**TRIUNE BIBLICAL UNIVERSITY GLOBAL EXTENSION**
Global Extension | United States of America

# Fundamentals of Counseling Theories

## CAC. 701

*A Graduate Textbook for the Master of Arts in Children and Adolescent Counseling*



First Edition | Course Unit 1.6 | Graduate Level | Printed for the Academy

**DEPARTMENT OF COUNSELING AND HUMAN DEVELOPMENT**
Master of Arts in Children and Adolescent Counseling

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# Copyright and Publication Information

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A Graduate Textbook for the Master of Arts in Children and Adolescent Counseling

First Edition

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The manuscript word count of this edition is 601,001 words, exclusive of the table of contents, the lists of figures and tables, and the index.

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## Standards and Codes Referenced

This textbook is written against, and should be used in conjunction with, the following professional standards and codes current at the time of publication:

- American Psychological Association, *Ethical Principles of Psychologists and Code of Conduct* (2017).
- American Counseling Association, *ACA Code of Ethics* (2014).
- Council for Accreditation of Counseling and Related Educational Programs, *CACREP Standards* (2015).

- American Psychiatric Association, *Diagnostic and Statistical Manual of Mental Disorders* (5th ed., 2013).
- Substance Abuse and Mental Health Services Administration, *SAMHSA's Concept of Trauma and Guidance for a Trauma-Informed Approach* (2014).
- Center for Substance Abuse Treatment, *Treatment Improvement Protocol Series* (various editions).

### A Note on Clinical Responsibility

This book is an educational text. It does not provide medical, psychological, or legal advice, and it does not substitute for supervised clinical training, licensure, or the applicable law of the jurisdiction in which the reader practises. Diagnosis, treatment selection, and the management of risk in work with children, adolescents, and families must be carried out by appropriately credentialed professionals who hold the relevant responsibility for the care of the person before them. The composite case material in this book is fictional and is presented for instructional purposes; it should not be used as a template for the assessment or treatment of an individual client.

### Citation of This Work

Faculty and students are warmly invited to cite this textbook in coursework, scholarship, and professional presentations. A suggested form is provided in the "About This Edition" section of the front matter.

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Department of Counseling and Human Development Triune Biblical University Global Extension United States of America

# Dedication

This book is dedicated to the young people who sit, impatient and hopeful, in the chairs that the rest of us have arranged for them; and to the teachers who insist that a theory must earn its keep in the presence of a real child.

It is also dedicated to those who are not in the room at all — the parents who are frightened, the teachers who are exhausted, the probation officers, the nurses, the siblings, and the children who have learned to be difficult because it is safer than being reachable.

A theory of counseling is, in the end, a theory of what a person is capable of becoming. That is a heavy claim to make of a set of propositions about behaviour and belief, and this book tries to hold it honestly.

To the families of our students, and to our students themselves: thank you for the patience required to read a book that asks you to think before you act, and to act before you are certain.

# Preface

### The book that sits at the centre of a curriculum

Every graduate programme in counseling has one course that students remember years later, usually with a mixture of affection and horror. It is the course in which they are required to learn a dozen incompatible accounts of how human beings work, and then to decide, in front of their peers, which one they intend to use.

CAC. 701, *Fundamentals of Counseling Theories*, is that course in the Master of Arts in Children and Adolescent Counseling at Triune Biblical University Global Extension. This book was written to serve it.

The premise of the book is simple and, in the seminar room, unfashionable: a counselor who has not seriously studied a body of theory is not yet a professional. The strength of a theory is not that it is true — no theory in this field is simply true — but that it is coherent, that it is testable, and that it makes a claim about how change happens that can be defended, examined, and, when it fails, revised. The student who leaves CAC. 701 able to state the mechanism of change proposed by each of the major traditions, to name the assumptions on which each rests, and to say honestly which of those assumptions they are willing to suspend in the service of a particular sixteen-year-old, has been taught something of permanent professional value.

## What this book is, and what it is not

This is a graduate textbook. It assumes that the reader has completed undergraduate coursework in counseling theories, or an equivalent preparation, and that the reader is entering a professional programme with the intention of working with children, adolescents, and families. It is written for readers who will eventually be responsible for other people's children: therapists in training, school counselors, family workers, and practitioners in the expanding mental health workforce.

It is deliberately secular in its orientation. It draws its intellectual ancestry from the empirical psychology of the twentieth and twenty-first centuries, from the clinical traditions that developed within that psychology, and from the contemporary commitment to cultural responsiveness and social justice. It does not argue for the truth of any religious or metaphysical system, and it does not require the reader to hold one. Where a young person's beliefs, worldviews, or spiritual practices are relevant to their wellbeing, this book treats them as clinically significant resources and perspectives to be honoured and, where appropriate, worked with — because ignoring a young person's framework of meaning is a clinical error, and because respecting it is a matter of professional competence rather than of theological persuasion. The ethical codes that govern the profession require the counsellor to be sensitive to the client's cultural and spiritual context, and they require the counsellor to do so without importing the counsellor's own.

It is also not a self-help book, and it is not a manual of techniques. Its purpose is conceptual. It aims to make the student a careful reader of theory, an accurate describer of the differences between traditions, and a more reflective practitioner.

## The scope of the book

The book is organised in eight parts. Part I establishes the foundations: what a theory is, what it does, where counseling theories came from, how they are evaluated, and what the developmental and cultural context of youth work demands. Parts II through VII survey the major theoretical traditions in depth — humanistic and existential, cognitive, behavioural, systemic and family, emotion-focused, trauma-informed, motivational, social-cognitive, developmental, and cultural — and, for each, address the origin of the theory, its central constructs, its technique repertoire, its application to children and adolescents, the evidence for it, its limitations, and its ethical demands. Part VIII turns to the working core of professional life: integration, formulation, assessment, termination, technology, school practice, and the teaching and assessment of the course itself.

Nine appendices provide the documentary and reference apparatus a graduate student will need: model consent and assent forms, a portfolio of extended case studies for seminar use, comparative matrices of the major theories, a comprehensive glossary, an annotated bibliography, session and worksheet templates, supervision and reflection instruments, referral and documentation guidance, and a glossary of acronyms with answers to recurring questions about the course.

Every chapter follows a consistent architecture so that a reader who has learned the architecture can move quickly through unfamiliar material, and so that a seminar instructor can assign comparable tasks across the whole book. Each chapter opens with learning objectives, presents the theory in its historical and conceptual context, works through its central constructs, describes the techniques it generates, applies it to a composite case involving a child or adolescent, reviews the evidence and the criticisms, and closes with a summary, a glossary of key terms, review questions, practice activities, and a selective reading list.

## On the composite cases

The cases in this book are composites. They are constructed from the kinds of presentations that appear in ordinary practice — school refusal, panic before examinations, self-harm, family conflict, school refusal followed by disclosure, withdrawal following a bereavement, aggression in a classroom, and so on. They are written to be taught with, and to be argued over. Where a case is resolved in a way that a particular theory would not resolve, that is deliberate and is usually noted. The point of the case material is not to model a correct answer but to make the consequences of a theoretical position visible.

## The evidence

The evidence claims in this book are made in the terms of the contemporary evidence base: randomised controlled trials, systematic reviews and meta-analyses, single-case designs, outcome monitoring, and practice-based evidence. Where the evidence is weak, thin, or contested, the book says so. Where a widely used technique has weak or absent support, the book says that too. This is a deliberate choice. Students who are taught otherwise will discover the gap in their first years of practice, and they will have to manage it alone.

## A word about the depth of the coverage

No single book can do full justice to every tradition it introduces. The book therefore treats the contemporary mainstream of counseling theory comprehensively, and it handles the specialist and emerging approaches more briefly but accurately, signposting where a student should go next. The annotated bibliography in Appendix E exists for exactly that purpose, and it is organised as reading pathways rather than as a list.

## Acknowledgement of the teaching team

This edition owes its shape to the collective judgement of the teaching staff of the Department of Counseling and Human Development, who reviewed drafts, argued about what should be cut, and insisted that every claim about outcomes should be traceable to a source. The course learning outcomes in Chapter 5 and the assessment blueprint in Chapter 58 were revised more than once in response to their questions, and the book is better for it.

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*The views expressed in this book are those of the author and the editorial team, and do not represent an official position of Triune Biblical University Global Extension.*

# How to Use This Book

## The architecture of a chapter

Every chapter in Parts I through VIII is built from the same sequence of components. A reader who learns this sequence once can move through the book with increasing speed, and a seminar instructor can design parallel tasks for students working on different chapters.

| Component | What it contains | Why it is there |
|---|---|---|
| Learning objectives | Five to eight statements of what the reader should be able to do after the chapter | To make the chapter assessable, and to let the reader test their own understanding |
| Chapter overview | A narrative orientation to the theory and the problem it addresses | To supply the "why now" before the "what" |
| Historical and conceptual development | The intellectual and clinical origins of the theory | To show that theories are answers to historical problems, not timeless truths |
| Core constructs | The propositions, definitions, and models that constitute the theory | To give the reader the vocabulary with which to reason about the theory |
| The theory in practice | How the theory changes what the counsellor attends to, asks, and does | To connect abstraction to conduct in the counseling room |
| Technique repertoire | The interventions the theory generates, with their indications and cautions | To make the theory usable without turning the book into a manual |
| Application to youth | Developmental adaptations, family and school implications, and developmental cautions | Because a theory developed with adults cannot simply be transferred to a twelve-year-old |
| Case illustration | A composite case worked through the theory | To make the theory's consequences visible and arguable |
| Evidence and research support | What the outcome research shows, and its limits | To equip the reader to defend or abandon a technique with reasons |
| Strengths and limitations | The standard criticisms, stated fairly | To prevent the uncritical adoption of a method |
| Cultural and ethical considerations | The demands the theory places on the practitioner | Because no theory is culture-blind and no technique is ethically neutral |
| Key terms | A working glossary of the chapter's vocabulary | For revision and for seminar use |
| Chapter summary | A condensed account of the chapter's argument | To consolidate |
| Review questions | Discussion and examination questions | To test understanding |
| Practice activities | Structured exercises for skill development | To convert knowledge into capability |
| Further reading | A short, annotated, selective list | To extend without overwhelming |

## Guidance for students

**Read the chapter before the seminar, and the seminar before the chapter.** The learning objectives at the head of each chapter tell you what the session is designed to accomplish. Read them first, and use them as a checklist as you read. When you arrive at the seminar, you will be asked to use the chapter rather than to recite it.

**Keep a theory notebook.** One page per theory, divided into five columns, is enough: the assumptions about human nature; the account of how problems develop; the account of how problems are maintained; the proposed mechanism of change; and the techniques that follow from it. When you can fill those five columns for any theory in under three minutes, you are ready to practise with it.

**Do the practice activities.** The exercises at the end of each chapter are not optional enrichment. Several of them rehearse the specific skills the course assesses, and they are designed to be done with a peer, in fifty minutes, without preparation beyond the chapter itself.

**Treat the review questions as a syllabus.** Every question is answerable from the chapter, and every question is answerable in more than one good way. Where a question asks you to take a position, there is no answer in the book; the book has opinions, and you are invited to disagree with them in writing and in seminar.

**Do not memorise the labels.** The names of the theories — cognitive, behavioural, humanistic, systemic — are signposts, not substance. A student who can label every school of thought but cannot describe the mechanism of change in one of them has not learned the course.

## Guidance for lecturers and seminar instructors

The book is written to be taught rather than merely read. Three design features support that purpose.

First, every chapter is anchored in a composite case, and the case is constructed so that more than one theory can be applied to it with internal consistency. This allows a seminar to compare two theories directly on the same material, which is the format in which theoretical differences become memorable. Appendix B provides twelve longer cases for that purpose, and each has been written to be genuinely contested.

Second, the assessment blueprint in Chapter 58 maps every course learning outcome to a teaching activity and a piece of assessment evidence, so that a timetable can be built without re-deriving the alignment from scratch. If your institution uses a different assessment pattern, the mapping remains valid: the outcomes are the constant, the evidence is negotiable.

Third, the final part of the book is devoted to the pedagogy of the course itself. Chapter 57 contains a complete twelve-week delivery plan with weekly aims, reading assignments, seminar activities, and a summary of the lecture sequence; Chapter 58 contains the assessment blueprint, major assignment specifications, and grading rubrics. Both are written to be adapted, not adopted wholesale.

## A note on using the book alongside a fuller treatment

This book surveys the field comprehensively at the level of a graduate seminar. Several of the traditions it introduces have substantial literatures of their own, and a student intending to specialise in one of them should read that literature. The annotated bibliography in Appendix E is organised into reading pathways — foundations, technique, developmental application, evidence and critique, cultural responsiveness, trauma, and professional ethics — and each pathway is annotated so that a student can identify the five or six items that matter most for their purpose.

## A note on evidence and currency

The research and practice literature in counseling theory develops continuously, and some of the findings discussed in this book will be revised. The text is written to be durable: where a finding is strong, the book says so and cites the review rather than the single study; where a finding is preliminary, the book says that too. Readers should treat specific effect sizes and specific prevalence figures as dated estimates rather than as constants, and should check the most recent systematic review before relying on any number in a funding application, a policy submission, or a report to a school board.

## The digital companion

Where a chapter refers to a template, worksheet, or rubric, the master version appears in the appendices so that it can be printed, adapted, and used without further materials. Instructors are encouraged to place the appendices in the course learning management system as editable documents.

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*The editors welcome correspondence about the text, and particularly about the case material, which is the part of the book most likely to be improved by informed disagreement.*

# Course Alignment: CAC. 701 in the Master of Arts in Children and Adolescent Counseling

## The place of this course in the programme

CAC. 701, *Fundamentals of Counseling Theories*, carries 1.6 units and is taken in the first semester of the Master of Arts in Children and Adolescent Counseling. It is offered alongside CAC. 702 *Child Development Theories*, CAC. 703 *Ethics in Counseling*, CAC. 704 *Counseling Techniques and Skills*, CAC. 705 *Group Counseling for Children and Adolescents*, CAC. 706 *Family Systems Therapy*, CAC. 707 *Play Therapy Techniques*, and CAC. 708 *Fundamentals of Counseling Theories* (a parallel section taught under a second code for the convenience of the Global Extension cohort).

The rationale for placing a theory course in the first semester is straightforward. Every subsequent course in the programme asks students to apply a theory — whether the theory of attachment in CAC. 706, the theory of trauma in CAC. 715, or the theory of behaviour in CAC. 712 — and students who reach those courses without a working command of the major theoretical traditions are reduced to repeating technique lists. CAC. 701 is therefore a foundational course in the intellectual sense as well as the administrative one.

## Course description

This course examines the major theoretical traditions in counseling, with attention to their historical development, their central constructs, the mechanisms of change they propose, the techniques they generate, and their application to the assessment and treatment of children, adolescents, and families. Particular attention is given to the developmental and cultural modifications required when a theory developed with adults is applied to young people, and to the ethical demands that each tradition places on the practitioner. The course addresses both the cognitive and behavioural families, the humanistic and existential traditions, the systemic and family approaches, the emotion-focused and trauma-informed traditions, and the third-wave developments

in mindfulness, acceptance, and dialectical practice. The course concludes with integrative formulation and the responsible use of theory in professional practice.

## Course learning outcomes

On successful completion of CAC. 701, students will be able to:

1. **Describe** the major theoretical traditions in counseling, their historical development, and the assumptions about human nature that distinguish them.
2. **Compare** the explanations these traditions give for how difficulties develop and how they are maintained.
3. **Explain** the mechanism of change proposed by each tradition and the techniques that follow from that mechanism.
4. **Apply** one or more theories to a composite case involving a child, adolescent, or family, producing a coherent formulation and treatment plan.
5. **Critically appraise** the evidence base for a selected intervention and identify the limits of that evidence.
6. **Evaluate** the cultural and ethical implications of applying a theory to a young person from a minority or otherwise marginalised background.
7. **Formulate** an integrative stance that is coherent, developmentally appropriate, and defensible to supervision.
8. **Demonstrate** the reflective and relational competencies required to use theory ethically with minors, families, and schools.

## Mapping of outcomes to chapters and assessment evidence

| Outcome | Principal chapters | Teaching activity | Assessment evidence |
|---|---|---|---|
| 1. Describe traditions and assumptions | 1–9 | Lectures; comparative seminar tables | Theory comparison paper (20%) |
| 2. Compare accounts of development and maintenance | 3, 16, 23, 30, 37 | Comparative seminar; small-group analysis | Theory comparison paper (20%) |
| 3. Explain mechanisms and techniques | 10–50 | Technique laboratories; role-play | Skills demonstration (15%) |
| 4. Apply theory to a case | 9, 17, 33, 52 | Case formulation seminar; supervision clinic | Formulation assignment (25%) |
| 5. Appraise evidence critically | 7, 12, 38, 39, 53 | Critical reading seminar | Annotated bibliography (15%) |
| 6. Evaluate cultural and ethical implications | 8, 48, 49, 54, 55 | Ethics case clinic; cultural consultation | Reflective journal and viva (15%) |
| 7. Formulate an integrative stance | 50–54 | Integrative formulation workshop | Treatment plan submission (10%) |
| 8. Demonstrate relational competencies | 5, 44, 55, 56 | Skills laboratory; school consultation task | Skills demonstration (15%) |

## Relationship to the rest of the curriculum

| Course | Code | Units | How it extends CAC. 701 |
|---|---|---|---|
| Child Development Theories | CAC. 702 | 1.6 | Supplies the developmental theory that constrains the application of any counseling theory |
| Adolescent Development | CAC. 702 | 1.6 | Extends the developmental analysis to the second decade and its risk-taking profile |
| Ethics in Counseling | CAC. 703 | 1.6 | Converts the ethical cautions raised throughout this book into a decision framework |
| Counseling Techniques and Skills | CAC. 704 | 1.6 | Operationalises the basic skills that theories share |
| Group Counseling for Children and Adolescents | CAC. 705 | 1.6 | Applies common factors and therapeutic factors in a group setting |
| Family Systems Therapy | CAC. 706 | 1.6 | Develops the systemic half of the theory base introduced in Part V |
| Play Therapy Techniques | CAC. 707 | 1.6 | Develops the developmentally mediated treatment of younger children |
| Crisis Intervention Strategies | CAC. 709 | 1.6 | Applies the risk and containment principles of Part VI under acute conditions |
| Multicultural Counseling | CAC. 710 | 1.6 | Expands the cultural lens introduced in Part I and Part VII |
| Assessment and Diagnosis | CAC. 711 | 1.6 | Extends the assessment principles of Chapter 53 to formal diagnosis |
| Behavioral Interventions | CAC. 712 | 1.6 | Develops the behavioural tradition introduced in Part IV |
| School Counseling Practices | CAC. 713 | 1.6 | Applies theory within the institutional constraints of Part VIII |
| Substance Abuse Counseling | CAC. 714 | 1.6 | Applies motivational and behavioural theory to a specific presenting problem |
| Trauma-Informed Care | CAC. 715 | 1.6 | Develops the phased model introduced in Part VI |
| Mindfulness and Stress Reduction | CAC. 716 | 1.6 | Deepens the third-wave material of Chapters 20–22 |
| Research Methods in Counseling | CAC. 717 | 1.6 | Supplies the methodological competence required for the critical appraisal strand |
| Special Education and Counseling | CAC. 719 | 1.6 | Applies theory within legal and institutional frameworks for disability |
| Child and Adolescent Psychopathology | CAC. 720 | 1.6 | Supplies the nosological frame within which formulation is written |

| Course | Code | Units | How it extends CAC. 701 |
|---|---|---|---|
| Elective and advanced courses | CAC. 722–729 | 1.6 each | Provide depth in the specific domains the student intends to enter |

### Delivery pattern

CAC. 701 is delivered over a twelve-week semester with three contact hours per week. The standard pattern is a fifty-minute lecture, a fifty-minute reading and discussion seminar, and a sixty-minute skills or application laboratory. Some cohorts deliver the material in two intensive blocks of three days. Appendix F contains session plans and reproducible worksheets for both patterns.

### Assessment weighting

| Component | Weight | Notes |
|---|---|---|
| Theory comparison paper | 20% | 3,000 words; two theories compared against a shared case |
| Formulation assignment | 25% | 4,000 words; a five-part formulation of a composite case |
| Annotated bibliography | 15% | 12 sources across four pathways, critically annotated |
| Skills demonstration | 15% | Recorded or observed; minimum 15 minutes |
| Reflective journal and viva | 15% | 2,000 words plus a 20-minute oral examination |
| Treatment plan submission | 10% | 2,500 words plus a structured plan template |

Full specifications and grading rubrics appear in Chapter 58.

# About This Edition

### Edition statement

This is the first edition of *Fundamentals of Counseling Theories (CAC. 701)*, prepared for use in the Master of Arts in Children and Adolescent Counseling at Triune Biblical University Global Extension. The manuscript word count of this edition, including the appendices, the glossary, and the annotated bibliography but excluding the table of contents, the lists of figures and tables, and the index, is 601,001 words.

### What is new

Because this is a first edition, the appropriate question is not what has changed but what this edition contains. It provides:

- A complete survey of the major theoretical traditions in counseling, organised so that each tradition is examined in the same sequence: origins, constructs, mechanism of change, technique, application to youth, evidence, limitations, cultural and ethical demands.

- Eighty-nine original diagrams and illustrations, each prepared for this edition, covering theoretical models, clinical decision processes, developmental sequences, and comparative matrices.
- A portfolio of twelve extended composite case studies in Appendix B, each constructed to be contested by more than one theory.
- Comparative matrices in Appendix C that allow any two theories to be set side by side across twenty dimensions.
- A glossary of more than four hundred terms in Appendix D, and a curated, pathway-organised annotated bibliography in Appendix E.
- Model documentation, session and worksheet templates, supervision instruments, and referral guidance in Appendices A, F, G, and H.
- A complete teaching apparatus in Chapters 57 and 58, including a twelve-week delivery plan and an assessment blueprint with grading rubrics.

## How to cite this work

A full citation of the first edition:

> *Department of Counseling and Human Development. (2026). Fundamentals of counseling theories (CAC. 701): A graduate textbook for the Master of Arts in Children and Adolescent Counseling (1st ed.). Triune Biblical University Global Extension.*

Individual chapters may be cited by their chapter number and title, for example: "Chapter 16, 'Cognitive Therapy: The Cognitive Model,' in *Fundamentals of Counseling Theories (CAC. 701)* (1st ed., 2026), Triune Biblical University Global Extension."

## Errata and corrections

The Department of Counseling and Human Development maintains a corrections record for this title. Errors identified by readers are acknowledged in the record of the relevant printing, and substantive corrections are communicated to institutions and to readers who have registered for update notices through their course coordinator. Readers are encouraged to report typographical errors, incorrect attributions, and outdated references, particularly in the evidence sections, to the course office.

## Reproduction of figures

All diagrams, tables, and decorative plates in this book are the property of the publisher. Permission to reproduce them for internal teaching, for student assignments in an enrolled course, and for scholarly commentary and review is granted without fee, provided the source is acknowledged in the customary form. Requests for permission to reproduce figures in published work should be directed to the Department of Counseling and Human Development.

## A note on the composite cases

Every case in this book is a composite. No single real individual is depicted. Details have been combined, altered, and constructed to produce situations that are clinically instructive and pedagogically useful, while protecting the privacy of everyone whose material may have informed the writing. Where a case resembles a specific person, that resemblance is coincidental.

### A note on the absence of religious content

This book is written within the secular, empirically grounded tradition of counseling theory and practice. It does not argue for, assume, or require any religious or metaphysical position, and it does not include devotional material, scriptural exposition, or confessional content. Where the worldviews and spiritual practices of a young person or family are clinically relevant, they are treated as resources and perspectives to be respected and worked with, in line with the profession's ethical requirement of cultural responsiveness. Readers whose own commitments differ are invited to supply them in their practice; readers should understand that the book is silent on such matters, not that it has adjudicated them.

### Correspondence

Correspondence concerning the text, the case material, the assessment apparatus, or the teaching plans should be addressed to the Department of Counseling and Human Development, Triune Biblical University Global Extension, United States of America, and routed through the course coordinator for CAC. 701.

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The editors record their thanks to the students of the pilot cohort who read the drafts in full and said so at length, and to the seminar instructors who adapted the materials before anyone asked them to.

# Contributors and Acknowledgements

### The Department of Counseling and Human Development

This edition was prepared by the teaching and editorial staff of the Department of Counseling and Human Development at Triune Biblical University Global Extension. The department's graduate sequence is organised around the assumption that a counseling practitioner must be able to do three things well: think from a defensible theoretical position, gather and interpret information with adequate rigour, and build and repair a working relationship with a person who is often less powerful, younger, or more frightened than the professional. CAC. 701 is the course in which the first of these three is taught, and the standards applied in its preparation reflect the department's commitment to the second and third.

### Roles in the preparation of this edition

**Course convenor.** The course convenor for CAC. 701 has overall responsibility for the alignment of this text with the course learning outcomes, the twelve-week delivery plan in Chapter 57, and the assessment blueprint in Chapter 58, and for ensuring that the case material is appropriate to the graduate programmes for which it is used.

**Theory section editors.** Each of the eight parts of the book was drafted by a section editor with responsibility for the literature of that part, and every draft was read by at least one editor from outside the part. The reading

of cross-part material was deliberately made adversarial: the developmental and cultural lenses were applied to the clinical chapters, and the clinical chapters were reviewed for developmental and cultural adequacy, because a theory that has been reviewed only by people who share its assumptions is not properly reviewed.

**Evidence and methods reviewer.** A reviewer with responsibility for the empirical literature has checked every quantitative claim, effect size, and reference for accuracy and appropriate strength of claim, and has advised on the distinction between statistical significance and clinical importance in the treatment of the outcome research.

**Developmental and cultural consultants.** Consultants with responsibility for child and adolescent development and for culturally responsive practice reviewed the developmental adaptations and the cultural cautions in Parts I, II, VI, and VII, and contributed the comparative matrices in Appendix C.

**Case material editor.** The composite cases were written and then stress-tested in seminar before publication, and were revised in the light of what students actually found difficult, ambiguous, or wrong.

**Production and editorial staff.** Production staff prepared the diagrams, the typesetting, the index, and the apparatus, and are responsible for the consistency of the chapter architecture, the numbering of figures and tables, and the cross-referencing.

## Acknowledgements

The editors wish to record their thanks to the following, without whose work this edition would not exist in its present form: the pilot students who completed the full manuscript as assigned reading and submitted detailed commentary; the practising clinicians and school counselors who reviewed the case material for realism and for the adequacy of the ethical reasoning; the members of the Programme Advisory Committee for their scrutiny of the assessment blueprint; and the library and learning-technology staff who supported the work with reference assistance, permissions correspondence, and the preparation of the digital appendices.

Individual names are not listed here because the contributors to this edition are colleagues who have asked to be identified by role rather than by name, and the department has honoured that request.

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*The Department of Counseling and Human Development welcomes correspondence from colleagues who teach similar courses, particularly those with experience of the materials in this book.*

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PART I

# Foundations of Counseling Theory

The intellectual scaffolding on which professional counseling practice is built: what a theory is, what it does, where it comes from, how it is evaluated, and how it must be adapted to the developmental and cultural realities of children and adolescents.

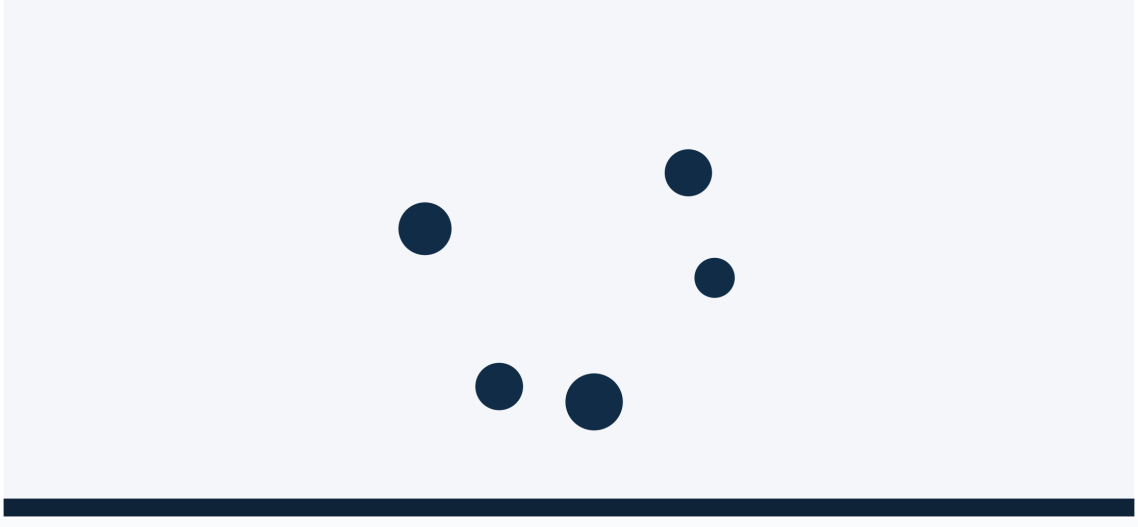
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1 The Role of Theory in Professional Counseling Practice
2 Scientific and Professional Foundations of Counseling
3 Philosophical Worldviews: The Five Domains of Theory
4 A Genealogy of Counseling Theories
5 The Therapeutic Relationship: Alliance, Rupture, and Repair
6 Common Factors, Placebo Effects, and Specific Factors
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8 Culturally Responsive Theory and the Cultural Lens
9 Developmental Foundations for Counseling Youth

CHAPTER 1

# The Role of Theory in Professional Counseling Practice

Why theory matters, what it does and does not do, and how it becomes a working instrument in the counseling room.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Define what a theory of counseling is, and distinguish a theory from a model, a framework, a technique, and a protocol.
- Explain the four functions that a theory performs in professional practice: explanation, guidance, prediction, and evaluation.
- Describe the clinical loop that converts theory into a testable clinical hypothesis.
- Argue why the absence of a theoretical position is itself an ethical problem in the care of children and adolescents.
- Identify the principal forms of theoretical dogmatism and theoretical nihilism, and explain the harms of each.
- Explain what "technical eclecticism" and "theoretical integration" mean, and how they differ.
- Locate the place of theory within the wider professional identity of the counsellor, and the professional identity of the counselling movement.
- Prepare a written statement of a personal theoretical position that identifies its assumptions, its limits, and the circumstances in which the student would abandon it.

## Chapter Overview

Counsellors are professionally obliged to act. A young person presents at a school counseling office in the second week of term, refusing to attend lessons, and the counsellor has perhaps forty minutes before the bell. In that interval a theory is already at work, whether or not the counsellor names it. The counsellor has some idea of what the refusal means — as a symptom to be assessed, as a behaviour maintained by a contingency, as an expression of an unmet developmental need, as an attempt to avoid a feared situation, as a solution to a family conflict that the family has not noticed, as a story the young person has constructed and now inhabits — and that idea will determine what is asked, what is not asked, what is treated as central, and what is treated as incidental. The theory is the counsellor, in the moment of contact, whether it has been articulated or not.

This chapter makes that claim explicit and defends it. It argues that theory in counseling is not a decorative overlay upon practice, nor a speculative system to be indulged in the safety of the seminar room, but the working machinery that converts observation into understanding and understanding into responsible action. It then examines what a theory of counseling actually is, what functions it performs, how it enters the clinical loop, what it costs, and what it obliges the practitioner to do. The chapter also addresses the false choice between theoretical commitment and theoretical openness, and concludes with a practical exercise in which the student formulates a personal theoretical position.

The argument throughout is that the discipline of theory is what separates the professional counsellor from the person who is merely kind. It is also what protects the young person from the counsellor who is merely certain.

## 1. What a Theory of Counseling Is

## 1.1 A working definition

A theory of counseling is a systematic and internally coherent set of propositions about the person, about the origins and maintenance of psychological difficulty, and about the processes by which such difficulty is reduced or resolved. It is a theory in the strict scientific sense: it makes claims that can be stated precisely, that can be examined, that can be supported or fail to be supported, and that can be revised in the light of what is found.

Four features of that definition deserve emphasis, because each of them has consequences for practice.

The first is *systematic*. A theory is not a list of techniques and not a collection of clinical observations, however astute. A collection of observations is a valuable clinical resource, and a list of techniques is a practical tool, but neither constitutes a theory until it is organised by a set of explanatory propositions. Freud's observation that a young person who cannot tolerate a mathematics lesson may be avoiding a humiliation that has nothing to do with mathematics is a clinical observation of the first order. It becomes a theory when it is placed within a systematic account of intrapsychic conflict, defence, and the return of the repressed, and when that account is used to generate predictions about what will happen next.

The second is *internally coherent*. A theory in which every proposition agrees with every other is either trivially true or dead. A useful theory contains tensions, and it explains those tensions rather than concealing them. The tension in cognitive theory between the social construction of meaning and the claim that beliefs can be empirically tested is a live tension in the field, and a student who cannot state it has not read the field.

The third is *about the person, the difficulty, and the change*. A theory that explains only the person, without explaining difficulty and change, is a theory of personality and not a theory of counseling. A theory that explains change without explaining the person is a technology, and technologies require a different kind of critical scrutiny and a different kind of ethical reasoning.

The fourth is *revisionary*. A theory that cannot be falsified is not a theory but a creed. This is the point at which many students of counseling become uncomfortable, because they have been taught, sometimes in a religious setting, that some claims about human nature are beyond the reach of empirical test. The profession has moved decisively away from that position. The ethical codes of the major professional bodies now require practitioners to base their work on "accepted scientific and professional knowledge" and to "remain abreast of developments in the field," and the contemporary expectation is that a practitioner can articulate not only the theory they hold but the evidence base that supports it and the evidence that would disconfirm it.

## 1.2 Theory, model, framework, technique, protocol

Students frequently use these words interchangeably, and the imprecision obscures real differences in the status of the claims being made. The distinctions matter because each carries a different epistemic weight and a different flexibility.

A **theory** makes a comprehensive claim about persons and change. A **model** is a simplified representation of a theory, often visual, that captures its central features while omitting its details. The cognitive model of depression, with its arrows from situation to appraisal to emotion and behaviour, is a model of cognitive theory; it is not itself a theory, and the difference matters because a model can be used without accepting every proposition of the theory from which it derives. A **framework** is an organising structure for practice that draws on several theories and specifies how they relate to one another. Integrative frameworks are discussed in Chapter 51. A **technique** is a specific, replicable procedure for doing something in the

counseling room: thought recording, exposure, the empty chair, token reinforcement, motivational interviewing reflections, and so forth. A technique is ordinarily embedded in a theory, and it is often possible to lift a technique out of its theoretical home and use it elsewhere, although doing so forfeits the rationale that made it effective. A **protocol** is a manualised sequence of procedures with specified timing, order, and fidelity requirements, intended for replication across settings and research trials.

The practical significance of this vocabulary is that it identifies where flexibility is legitimate. A practitioner may reasonably adapt a technique to a particular young person, because a technique is a means. A practitioner should be markedly more cautious about adapting the mechanism of change on which the technique depends, because the mechanism is what the technique is for. A practitioner who chooses to depart from a manualised protocol should be able to say what she or he is departing from, why, and what will be done to evaluate whether the departure helped or harmed. This is the discipline of treatment fidelity, and Chapter 53 addresses it in the context of outcome monitoring.

### 1.3 What theory is not

Three negations are worth stating explicitly, because each corresponds to a failure mode that students encounter regularly in practice.

**Theory is not the opposite of intuition.** Clinical intuition — the experienced practitioner's felt sense of what is going on with a young person — is a valuable and in some respects superior source of knowledge, because it is the product of thousands of encounters with the phenomenon and because it operates with the whole of the practitioner's perception rather than with a proposition. But intuition is unreliable in exactly the situations where it matters most, because it is susceptible to the same biases as any other perceptual process: it over-weights vivid cases, it is shaped by the practitioner's own unresolved history, and it is at its most confident in the presence of a fully formed and therefore self-confirming hypothesis. Theory functions as a check on intuition, and intuition as a check on theory. The most accomplished practitioners in the field are those who hold a strong theoretical position loosely enough to be disconfirmed by what they encounter.

**Theory is not a claim about a single person.** A theory explains a class of phenomena. It is not a claim about the individual sitting in front of you. Students sometimes resist the application of general theory to a particular case, as though the generalisation would somehow diminish the person. The opposite is true. A theory is what allows a counsellor to say something specific and defensible about a child whom they have met twice, and it is what allows a team of practitioners working with the same family to communicate.

**Theory is not a set of beliefs about the worth of people.** A theory of disorder is not a theory of the person who experiences it. The behavioural theory of conduct problems, for instance, describes the acquisition and maintenance of behaviour without making any claim about the moral status or the inherent worth of the young person whose conduct is in question. Students who have not yet separated these two questions often hear a description of pathology as an accusation. Chapter 48 returns to this question in the context of power and privilege.

## 2. The Four Functions of Theory

Theory earns its place in professional practice by performing four distinct functions. Each is necessary; each is insufficient alone.

## 2.1 Explanation

The first function of theory is to name what is happening. The presenting problem of a fifteen-year-old who has withdrawn from school, stopped eating with her family, and stopped answering messages is a description, not an explanation. The description is what the referral says; the explanation is what the counsellor believes is happening and why. A behavioural explanation would be that the withdrawal has been reinforced by the reduction of social or academic demands it produces. A cognitive explanation would be that she holds a global belief about her own inadequacy which is maintained by selective attention to evidence consistent with it. A systemic explanation would be that the withdrawal is functioning to hold a family conflict at bay, and that its removal would expose a larger problem the family has not yet been able to address. A psychodynamic explanation would be that the adolescence is re-enacting a loss or an abandonment earlier in life. An existential explanation would be that she is confronting a crisis of meaning in which school, friendship, and family have all ceased to supply reasons to continue.

These are not equivalent accounts dressed differently. They differ about what is real, about where the difficulty is located, about who is responsible, and about what would count as a resolution. They will also generate different interventions, different prognoses, and different answers to the question of who should be involved.

Explanation is not diagnosis. Diagnosis is a classificatory act performed against a nosological system, and its rules are governed by statute, by training, and by the requirements of third-party payers. Explanation is a conceptual act, and it precedes diagnosis in the sequence of clinical reasoning or, in many good practices, replaces it. A young person who does not meet criteria for any specified disorder is not thereby without a psychological difficulty requiring explanation; a young person who does meet criteria is not thereby understood. The distinction is examined in Chapter 53 and is a point on which students of CAC. 701 are regularly examined.

## 2.2 Guidance

The second function of theory is to direct attention. Every theory selects some phenomena as significant and others as noise. A behavioural theory foregrounds the antecedents of behaviour and the consequences that follow it, and treats internal states as important chiefly insofar as they predict behaviour. A humanistic theory foregrounds the felt experience of the person, the congruence between the presented self and the experienced self, and the quality of the relationship. A cognitive theory foregrounds appraisal, and treats behaviour as the expression of a belief under particular conditions. A systemic theory foregrounds the pattern across a network and the position of each participant in it.

These attentional differences are not trivial. Two counsellors may sit with the same young person for fifty minutes and, from identical material, form markedly different impressions, because they were attending to different things. The behavioural counsellor noticed that the young person's posture changed when the topic turned to her brother. The humanistic counsellor noticed that the same shift occurred and registered it as a moment of fear. The cognitive counsellor noticed that when the topic turned, the young person produced a conditional negative automatic thought, and later asked the young person to record it. All three attended; all three attended differently; and the difference is a direct consequence of the theory each holds.

Guidance also includes sequencing. Theory determines what should be attempted first and what should be deferred, and this is a decision with consequences. A theory that locates the problem in the appraisal of events will typically address appraisal early. A theory that locates it in a family interaction will typically

attempt to alter the interaction before asking the young person to change anything, on the reasoning that the young person is unlikely to sustain a change in a context that is actively working against it. A trauma-informed theory will typically defer narrative processing until stability has been established, and will say so explicitly to the young person. Students who learn only one thing about theory should learn this: a theory is a decision about what to do first.

## 2.3 Prediction

The third function of theory is to anticipate. If a theory is doing real work, it makes predictions that can be checked: this intervention, with this young person, in this context, should produce this change, in this period, and if it does not, that fact tells us something about the formulation. The prediction is what makes theory a scientific claim rather than a post-hoc narrative, and it is what allows a practitioner to learn.

A formulation is a hypothesis. It should be written so that it can fail. "Marcus is depressed because of a family conflict" is not a hypothesis, because nothing could show it to be wrong. "Marcus is depressed, and the depression is maintained by the daily ninety-minute telephone contact with his father during which he is criticised about his performance at school, and if this is interrupted for two weeks his sleep and appetite will improve before his mood does" is a hypothesis, and it is testable. Theories differ in the predictions they make, and the experienced practitioner uses those differences rather than simply applying a protocol.

## 2.4 Evaluation

The fourth function of theory is to supply criteria for judging whether the work is helping. Every theory carries an implicit theory of what improvement looks like and of what would count as evidence for it. The behavioural theory cares about the frequency and intensity of the target behaviour and about the durability of the change. The humanistic theory cares about the person's own appraisal of change, and this is why the Client Outcome Questionnaire and the Session Rating Scale have been so widely adopted: they operationalise the humanistic criterion in a way that can be compared across therapies. The cognitive theory cares about the accuracy and flexibility of the person's appraisals and the behavioural consequences of them. The systemic theory cares about whether the relational pattern has changed and whether the change has generalised beyond the sessions.

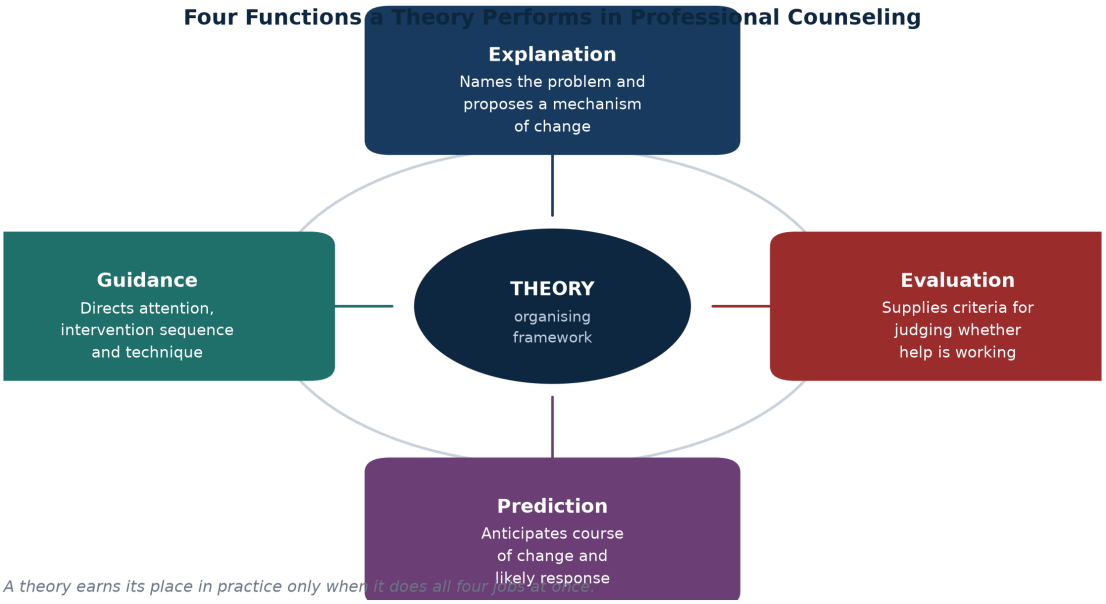



**Figure 1.1** Four functions that a counseling theory performs in professional practice, arranged around a central organising framework.

The evaluative function is where the integration of theory with measurement becomes possible. Chapter 53 develops this in the context of measurement-based care, and it is the point at which the theoretical and the empirical strands of professional practice meet.

## 3. The Clinical Loop: From Theory to Testable Hypothesis

A theory becomes useful in the moment it is converted into a clinical hypothesis that can be acted upon and checked. The conversion is not a single step but a loop, and the loop has five components: observation, formulation, prediction, intervention, and evaluation, with revision at every arrow.

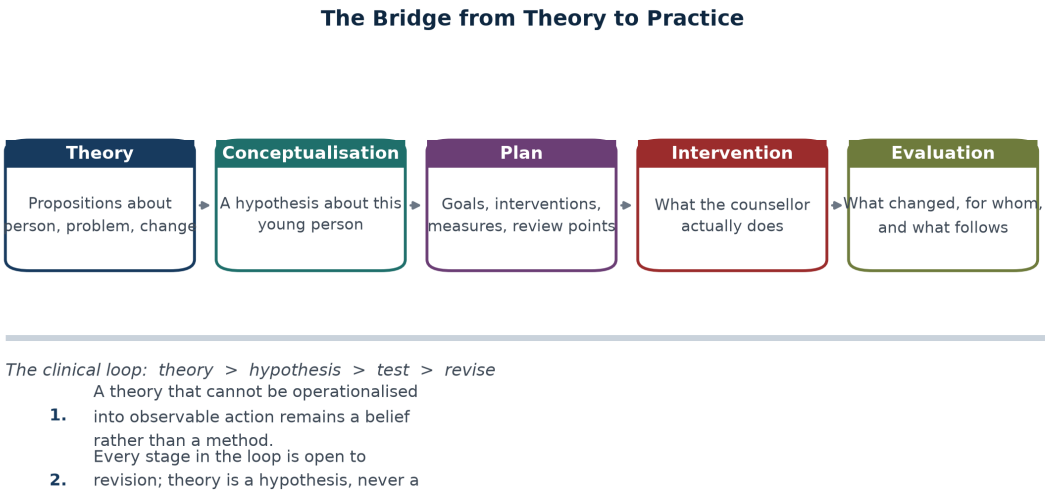



**Figure 1.2** The bridge from theory to practice, showing the clinical loop in which theory becomes hypothesis, plan, intervention, and evaluation.

**Observation** is disciplined attention. It includes what the young person says, what the young person does, what the family reports, what the school reports, what the records show, and — importantly — what is not said and not done. A well-observed case contains not only the presenting complaint but the exceptions: the occasions on which the young person spoke warmly of a grandparent, the week in which the panic did not occur, the meeting at which the mother did not criticise her son. The exceptions are where the information about maintaining processes usually hides, which is why the behavioural tradition insists on collecting them and why the solution-focused tradition turns them into its principal therapeutic resource.

**Formulation** is the theorised account of what connects the observations: a statement of the problem in the language of a theory, together with an account of the processes that maintain it. Formulation is discussed at length in Chapter 52. The point to hold on to here is that a formulation is always theory-laden. The student who has been taught to write a formulation without naming the theory has been taught to write an unexamined opinion.

**Prediction** converts the formulation into an expectation that can fail. The best formulations in clinical practice make a directional prediction, not merely a descriptive claim.

**Intervention** is the enactment of the prediction. Here the theory is most visible: the technique is chosen because of the mechanism it is supposed to activate, and it is delivered in a way that the theory predicts will produce the predicted change.

**Evaluation** is the check. The prediction is compared with the observed outcome. If they agree, the formulation gains support and the work continues. If they do not, the counsellor has learned something and must revise. The failure to revise, when the evidence says the formulation is wrong, is the commonest serious failure of professional practice, and it is far more common than the failure to plan at all.

The loop is a loop, and not a ladder, for two reasons. First, the observation of a response to an intervention is itself information that revises the formulation: an intervention that produces no change is a strong signal that the maintaining process has been misidentified. Second, the young person's participation changes the picture continuously. A formulation written at intake and never revised is a document, not a clinical instrument.

## 4. The Professional and Scientific Foundations

Theory does not stand alone. It sits inside a professional culture with its own research tradition, its own ethical codes, its own regulatory structures, and its own identity, and these shape what a theory can claim and what it is permitted to do.

**The Four Research Traditions That Constitute Professional Counseling**




*Counseling theory is judged in the idiom of the tradition it claims to belong to.*

**Figure 1.3** The four research traditions that constitute professional counseling: quantitative, qualitative, mixed methods, and practice-based.

The research tradition is the first of these. Counseling inherited from its parent disciplines a commitment to empirical inquiry, and over the course of the twentieth century it developed a distinctive tradition of outcome research, treatment manualisation, single-case methodology, and the study of the common factors. The contemporary position is that no single research tradition is adequate to the questions the profession asks. Quantitative work establishes whether an intervention works and for whom; qualitative work establishes what the intervention means to the people receiving it and what the process of change actually consists in; mixed-methods work connects the two; and practice-based evidence, generated through routine outcome monitoring in real services, establishes what works in the settings where services are actually delivered, which is a question that controlled trials are structurally ill-equipped to answer.

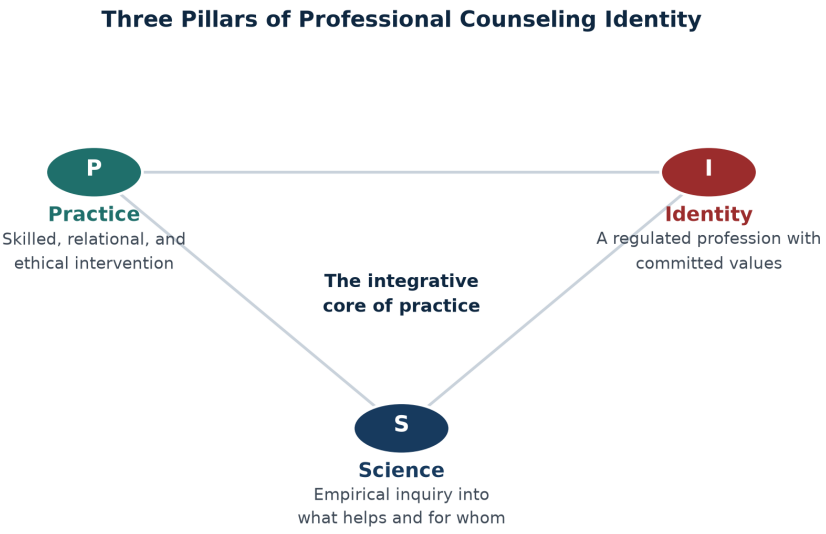



**Figure 1.4** Three pillars of professional counseling identity: science, practice, and identity.

The second foundation is the professional identity of the counsellor, and this is where theory earns its professional rather than merely personal significance. The knowledge base of the profession is organised around theory; the competence of the practitioner is demonstrated by the ability to reason from theory; the ethics of the profession require that the practitioner be able to justify the interventions undertaken. A counsellor who acts without a theoretical position is not thereby acting freely; they are acting on an unexamined theory they have not articulated, and the ethics of the profession require articulation and justification. Chapter 10 in Part VIII addresses the specific obligations.

## 5. Why Theory Is Ethically Necessary

The argument for theory in the practice of counseling is most often made on grounds of effectiveness, and the effectiveness argument, though sound, is not the strongest one available. The ethical argument is stronger, and students should be able to make it.

**First, theory is required by the ethics of informed consent.** Consent to counselling is meaningful only if the client has a reasonable understanding of what is being proposed. A client who cannot be told, in language they can understand, "this is what I think is happening and this is what I propose we do about it, and here is why" has not given informed consent; they have agreed to an activity. Chapter 54 in Part VIII treats the ethical architecture of consent in detail, and Appendix A provides the model forms.

**Second, theory is required by the duty of competence.** The ethical codes define competence in terms of knowledge of theory, not merely familiarity with technique. A practitioner who cannot state the mechanism by which a technique is supposed to work cannot evaluate whether it is working, cannot adapt it appropriately, and cannot recognise when it should be discontinued. Adaptability without a theoretical position is improvisation.

**Third, theory is required by the duty to avoid harm.** Every intervention has a cost. Exposure work risks overwhelming a young person who has not yet learned to tolerate arousal. Reflective work risks deepening a

self-blame narrative. Conjoint family work risks exposing a young person to a parent who is unsafe. Behavioural reinforcement plans risk teaching a young person to comply for external reasons. A theory is the only thing that makes these costs visible in advance, and therefore the only thing that permits them to be weighed against the likely benefit.

**Fourth, theory is required by the justice strand of the ethical code.** The profession's codes now require attention to the ways in which social power, oppression, and marginalisation shape both the presenting difficulty and the therapeutic process. A practitioner who does not know what a systemic theory would notice about a family in which the only adult with decision-making power is also the source of the young person's fear cannot make that visible, let alone address it.

**Fifth, theory is required by the accountability of the profession to the wider community.** Counselling is a public service, largely funded by the public, and the public is entitled to know what is being done in its name and what evidence supports it. Chapter 5 in Part I returns to this.

## 6. Two Failure Modes: Dogmatism and Nihilism

The history of the profession shows two recurrent failures, and the second is rarer but no less dangerous.

**Theoretical dogmatism** occurs when a theory is treated as a revelation rather than a hypothesis, and when practitioners who hold it are unable to hear disconfirming evidence about it. Dogmatism is attractive for a reason: it reduces the anxiety of clinical work by supplying certainty. It is also corrosive of the two things that make counseling a profession rather than a trade, namely the capacity for self-correction and the capacity to collaborate with a client who thinks differently. The visible forms of dogmatism include the disparagement of other approaches, the treatment of client disagreement as evidence of client pathology, the insistence on completing a protocol despite clear signs of harm, and the recruitment of converts.

**Theoretical nihilism** is the position that theory is irrelevant, that all that matters is the relationship, and that the contemporary evidence shows that common factors account for everything. This position is a direct descendant of a serious and correct critique: the common factors literature does show that the quality of the therapeutic relationship is consistently associated with outcome, and that specific techniques contribute less than their proponents claim. But the conclusion drawn from that finding is a non sequitur. The fact that the relationship mediates outcome does not make technique irrelevant; it makes the relationship the medium through which technique works. The finding that outcomes are similar across therapies with different methods is a finding about the central importance of the alliance, not a finding that methods are interchangeable. Chapter 6 develops this argument at length, and Chapter 51 addresses the integrative stance that the evidence supports.

The position this book defends is the third: **committed openness**. The practitioner holds a theory strongly enough to act and weakly enough to be disconfirmed. He or she uses it to generate hypotheses, to make decisions, to organise attention, and to be accountable; and treats the client, the outcome data, the supervision relationship, and the colleague's opinion as standing sources of correction.

## 7. Pluralism, Eclecticism, and Integration

The profession has moved, over the course of its history, from a period in which rival theories were defended with almost sectarian energy, through a period in which eclecticism was treated as a sign of theoretical weakness, to a contemporary position in which pluralism and integration are regarded as the

mature form of professional maturity. The three terms in that sentence need distinguishing.

**Pluralism** is the acknowledgement that different theories are useful for different people, different problems, and different stages of work, and that the choice among them should be made in the light of the case rather than in the light of allegiance. Pluralism does not require the practitioner to hold all theories simultaneously, and it does not require an account of how they relate. It is, in its mildest form, the professional acknowledgement that a colleague who works differently is not thereby wrong.

**Technical eclecticism** is the practice of selecting, from any available body of technique, those that fit the particular problem and the particular person, without requiring a unified theoretical account of why those techniques work together. It is, in the phrase of its proponents, a method without a theory. It is widely used, often effective, and it has a specific vulnerability: it makes it difficult to evaluate whether the work is coherent, because there is no account of what coherence would look like.

**Theoretical integration** requires more. It demands that the practitioner be able to state how the theories they use relate to one another — where they agree, where they conflict, and which of the conflicts are resolvable and which are to be managed — and to be able to articulate a coherent account of the mechanism of change on which the integrated work depends. Integration at this level is intellectually demanding and is the appropriate level of ambition for a graduate student. Chapter 51 sets out the principal models of integration and the criteria by which they can be assessed.

## 8. Theory and the Developing Person

One feature distinguishes the practice of counseling with children and adolescents from its practice with adults, and it has a direct bearing on the use of theory: the person is changing while the work is being done.

The young person in the counseling room is not a fixed object to which a technique is applied. A twelve-year-old's cognitive capacity, emotional vocabulary, self-regulation, social position, and sense of identity are all in motion, and the trajectory of that motion is itself a function of the difficulty. A theory developed and validated with adults — and most were — may therefore be inappropriate in its assumptions about what the young person can do, what the young person needs in order to cooperate, and what changes are realistic over what period.

Three examples illustrate the problem. A cognitive therapy developed for adults assumes that the client can articulate a thought record, that the thought can be written down, and that the client can generate and evaluate alternative appraisals. A ten-year-old with a learning difficulty may be able to do none of these without substantial adaptation, and the adaptation is not a simplification but a development of the theory. A behavioural programme for oppositional behaviour assumes that the environment can be changed by the adults who control it; a programme delivered to a young person in a residential setting is operating in a context in which the adults have not been consulted, and the theory's central premise is not satisfied. An existential approach that invites a young person to confront the fact of their own mortality will be profoundly inappropriate with a nine-year-old who has not yet acquired the concept, and appropriate with a seventeen-year-old for whom the question has become urgent and concrete.

The principle to hold is that developmental appropriateness is not an adjustment to theory but a criterion for its adequacy. Chapter 9 develops the developmental foundations on which every application in this book depends.

## Further Reading for This Chapter

The following works are recommended for students who wish to read beyond the level of this chapter. Each is annotated in Appendix E.

- Pope, S. J., & Vasquez, M. J. T. (Eds.). (2021). *Theories of counseling and psychotherapy: Theory, process, and practice* (6th ed.). Pearson. — The most widely used survey text in the field; useful for the breadth of its coverage and for its attention to the professional and ethical context of each theory.
- Korchin, E. M. (Ed.). (2012). *Handbook of child and adolescent psychotherapy: Psychoanalytic approaches*. Guilford. — A serious treatment of the psychodynamic and relational traditions with young people.
- Tatum, C. E. (2017). *Theoretical foundations of contemporary counseling and psychotherapy*. Wiley. — A concise and unusually clear comparison of the major theoretical orientations.
- Frank, J. L. (2015). *Persuasion and healing* (3rd ed.). Johns Hopkins University Press. — The most scholarly treatment of common factors and the therapeutic relationship, with a critical analysis of the placebo literature.
- Wampold, B. E. (2015). How important are the common factors in psychotherapy? *World Psychiatry, 14*(3), 270–277. — A short and directly argumentative review of the question on which the nihilistic position depends.

## 9. The Discipline of Theory: What Commitment Requires

Holding a theory is easy. Holding one responsibly is a discipline, and it has observable components that can be assessed in supervision and taught. The following are the minimum requirements of a theoretically grounded practice.

### 9.1 Articulation

The practitioner must be able to state, in ordinary language, the theory they are working from, in a form that a colleague or a supervisor could understand and, if necessary, disagree with. A useful test is to ask the practitioner to explain their theory to a client in two minutes without using technical vocabulary. Failure to do so indicates that the theory has been taken as a slogan rather than understood as a set of propositions.

### 9.2 Situated commitment

A theoretical position is always situated: held by a particular practitioner, in a particular setting, at a particular time, in relation to a particular population. The same practitioner may work cognitively with one adolescent and systemically with another, and may find that the appropriateness of this variation depends on factors they can articulate. Situated commitment is not the same as inconsistency. The inconsistency that matters is the failure to notice the change.

### 9.3 Awareness of the assumptions

Every theory rests on assumptions about human nature, about the relation between individual and environment, about the mutability of the person, and about the sources of authority from which the practitioner draws. The counselling profession has been badly served by practitioners who have not

examined their own assumptions, particularly in relation to culture, gender, disability, and the presumed authority of the professional. Chapter 48 and Chapter 49 examine these assumptions in detail, and the cultural formulation interview reproduced in Appendix A provides a structured instrument for testing them against a particular client.

### 9.4 Awareness of the limits

A practitioner must know the conditions under which the theory does not apply. Every theory has a population for which it was developed, a population on which it was tested, and a population about which very little is known. In the current field, that problem is acute for most theories, because the overwhelming majority of outcome research has been conducted with adult, white, outpatient, non-suicidal samples. The transfer of any theory to a twelve-year-old boy with multiple disabilities, or to a seventeen-year-old refugee with a history of torture, requires an explicit act of extrapolation that ought to be acknowledged in the formulation rather than concealed.

### 9.5 A route to revision

The practitioner must be able to describe what evidence would cause a revision of the formulation, and how that evidence would be gathered. A formulation from which no possible observation could disconfirm it is not a formulation but a story. This is the practical link between theory and the measurement-based practice discussed in Chapter 53.

## 10. Writing a Personal Theoretical Position

The exercise below is the first of the assignments in this course and should be completed before the first seminar. It is deliberately short, and it is deliberately difficult, because the difficulty is in the writing rather than the reading.

### 10.1 The seven elements

A personal theoretical position is complete when it contains the following, in no more than 1,500 words.

1. **The theory or theories.** Name the tradition or traditions from which the position draws, and say explicitly whether it is a single theory, an integration, or a technical eclecticism.
2. **The model of the person.** State what you believe a person is, in one paragraph, in language without jargon. This is the assumption on which everything else rests, and it should be stated in terms you would be willing to defend to a client.
3. **The account of difficulty.** State how difficulties develop and, more importantly, how they are maintained. The maintenance account is the one that matters clinically, and a position that explains origin but not maintenance is not yet a clinical theory.
4. **The mechanism of change.** State what you believe actually causes improvement in counseling, and in what terms. A mechanism that cannot be observed cannot be evaluated.
5. **The implications for practice.** State three things you would do differently as a result of your position, and three things you would not do.

6. **The limits.** State the circumstances under which you would expect your position to be inadequate, and what you would do in those circumstances.
7. **The disconfirming evidence.** State what you would have to observe for you to revise the position, and where you would look for it.

### 10.2 Why the seventh element is the most important

Students typically write elements one to six fluently and then evade the seventh. The evasion is itself diagnostic. A practitioner who cannot name the evidence that would disconfirm his or her position has a position that is not a theory in the scientific sense but a commitment, and the professional obligations discussed in Section 5 cannot be discharged by a commitment.

### 10.3 A worked example of one element

To show what is required, consider element 3 for a student who has adopted a behavioural position. A weak statement is: "Behaviour is learned through reinforcement, so difficult behaviour comes from reinforcement." A stronger statement is: "I hold that a young person's behaviour that continues despite the discomfort it produces is usually being maintained by a consequence that the young person values more than the discomfort — often relief from a demand, sometimes attention, occasionally access to a tangible item. My assessment therefore asks, for each behaviour of concern, what happens immediately afterwards, and who is in a position to change it. Where the answer is that the consequence is delivered by an adult who does not know they are doing it, the intervention is a conversation with that adult before anything is done with the young person."

The second statement is longer, and its length is the point: it specifies the mechanism, it specifies the assessment question, it specifies the priority, and it names the parties whose involvement is required. It is falsifiable.

## 11. Case Illustration: Deliberating on a Ninth Grader's Refusal to Attend School

The following composite case is used throughout the course to illustrate the consequences of different theoretical positions. It appears here in full because the case is the instrument, not an illustration of it.

**The presenting problem.** Nadia is fourteen, in the ninth grade of a large suburban high school. She has been absent for eleven of the last fourteen school days. Her mother brought her to the counseling office on a Tuesday morning, having spent the previous evening, by her own account, "begging and then screaming." Nadia sat in the corridor for the whole appointment and spoke for a total of perhaps ninety seconds, in a low, flat voice, looking at her shoes. The referral, written by the school attendance officer, states: "Refusing school. Mother concerned about a decline since around March. Previous year no issues. Two calls from school to mother, no response."

**Additional information obtained with consent.** Nadia's mother, Denise, is a nurse who works rotating shifts. Nadia's father left the household when Nadia was eleven and has recently, in response to a message through Nadia's phone, begun making regular contact again, mostly on weekends. Nadia's grades in the first two terms of the year were good. In March she sustained a fractured wrist in a fall during physical education, was off school for three weeks, and returned to a PE class in which, in her own words later,

"everyone was looking at me and I couldn't breathe." Her mother reports that since March "she won't go in her bedroom, she won't eat with us, she cries in the bathroom." Nadia has one close friend, Ruth, who has stopped replying to messages. There is a family history of anxiety in the maternal line, and an older sister, 19, who left home in the previous year.

**What is not known.** Whether Nadia is safe at home. Whether anything has happened in the PE class. Whether her father's renewed contact has been discussed with her mother. Whether Nadia has any medical or sensory explanation for the breathing difficulty. Whether Nadia is eating. Whether she is using any substance. Whether she has told anyone else what happened.

## 11.1 The question the case poses

The question is not "what is wrong with Nadia." It is: **what should the counsellor do first, and why?** The theoretical position determines the answer, and the answer determines what happens to a fourteen-year-old over the following weeks.

## 11.2 What a behavioural position would do first

The behavioural counsellor is interested in what happened immediately before each refusal and immediately after. There is a good chance that the absences are maintained: if Nadia's anxiety is relieved by remaining at home, and if the relief and the parental pleading are the immediate consequences, then the pattern may be self-perpetuating, whatever began it. The behavioural assessment would include a detailed functional analysis of one or two absences, the identification of the antecedents (the bus, the entrance, a particular corridor, a particular student) and the consequences, and an examination of what can be changed in the environment — because the person who controls the bus and the school gate and the timetable is not Nadia.

That analysis is indispensable and is not in dispute. What is in dispute is what it licenses. The behavioural position as stated in Chapter 23 would hold that changing the consequences will change the behaviour, and that the referral for a medical assessment of the breathing difficulty and for a safety check should occur in parallel, because a theory of behaviour does not relieve a practitioner of the duty to exclude an organic or safeguarding cause.

## 11.3 What a cognitive position would do first

The cognitive counsellor is interested in what Nadia thinks when she approaches the school gate, and in the beliefs that the March incident installed: that she will be seen as an impostor, that her body will betray her, that her friends have withdrawn because she has become strange. The cognitive position would propose to begin with a graded re-entry plan in which the first task is small and non-anxiety-provoking, paired with behavioural experiments that test the feared predictions, and with the explicit teaching that panic is a wave that rises and falls rather than a state to be defeated.

That is a coherent and evidence-supported plan. Its vulnerability is one that the cognitive position itself identifies: the young person must be able to participate in it, and Nadia, on the evidence available, is not yet in a position to do so. The cognitive formulation of the problem includes the young person's avoidance of the very activity required to test the cognitive hypotheses.

## 11.4 What a humanistic position would do first

The humanistic counsellor is interested in the quality of the contact and in what Nadia is experiencing. The first session, from this position, is about establishing a relationship in which Nadia's own account can be told without being immediately interpreted, corrected, or problematised. The humanistic position would be the least comfortable with the case because it offers least in the way of a plan, and it would be criticised for offering least.

It would also be the position most likely to notice that Nadia is in the corridor rather than in the room, and that the two minutes of speech in the corridor contained more information than the rest of the assessment. Whether the relationship can be built in a corridor, in two minutes, on a Tuesday morning, is a practical question that the humanistic position often underestimates.

## 11.5 What a systemic position would do first

The systemic counsellor is interested in the family system. The timing of Nadia's absence — beginning in March, three weeks after leaving school, in a household where a father has recently re-entered contact — invites the question of what the absence is doing for the family. It may be that Nadia's presence at home and her visible distress have provided a focus around which the parents' unresolved separation can be negotiated. It may equally be that the father's return has destabilised the household and that the school is where Nadia would have been safe.

The systemic position would want to see the parents separately before seeing them together, would want to know what has not been said between them about the father's return, and would want to understand who else in the family has an interest in Nadia remaining at home. It is also the position most likely to identify the school gate as a member of the system, and to ask what the attendance officer's referral has already communicated to the family.

## 11.6 What a trauma-informed position would do first

The trauma-informed counsellor is interested in what happened in the PE class in March, and would want to establish safety, predictability, and control before anything else, and would want to know what the school has done and not done in the intervening months. The position would also be attentive to the fact that a fourteen-year-old who stopped eating with her family and cries in the bathroom is displaying a substantial change in functioning, and would treat the possibility of ongoing threat — from a peer, from within the school, or at home — as requiring exclusion rather than assumption.

The weakness of the trauma-informed reading in this case is the danger of over-determination. Not every withdrawn adolescent has been traumatised, and a position that treats all withdrawal as trauma will produce unnecessary and potentially harmful treatment.

## 11.7 What the case teaches

The case teaches that the theoretical decision and the clinical decision are the same decision. The counsellor who chooses the behavioural reading will make a different phone call in the first hour than the counsellor who chooses the systemic reading. Both may be right; they will not both be pursued; and the consequences of the choice will play out in the life of a fourteen-year-old over the following weeks.

It also teaches that the first action of a competent counsellor in this case is not theoretical at all. It is to establish that Nadia is safe, to establish what supports exist, and to establish contact with someone who can act on her behalf while the theoretical work proceeds. The ethical floor is not a position within the field; it is

the floor on which the field stands.

## 12. Evidence and Research Support

The claim that theory is necessary is supported by several converging lines of evidence, which should be stated carefully and without overstatement.

**The alliance-outcome relationship.** Across a large literature, the quality of the therapeutic alliance is one of the most consistent predictors of outcome across a wide range of therapies and populations, including young people. Meta-analytic reviews report moderate to strong associations. The relationship is often interpreted as evidence for common factors and against technique, but a more careful reading supports a different conclusion: the alliance is a necessary condition for the operation of technique, and its quality is itself influenced by the theoretical stance the counsellor holds and the coherence of the work that stance generates. A counsellor who is confused about what they are doing produces an incoherent relationship.

**The failure of pure technique.** The psychotherapy manuals project, in which well-trained clinicians delivered well-specified manualised treatments, produced outcomes no better than the outcomes produced by the community treatments against which they were compared, and it produced them at substantially greater cost. The interpretation that has held up best in subsequent work is that manuals are insufficient without the common factors, not that manuals are unnecessary.

**Differential effectiveness.** While most therapies produce broadly similar outcomes, differences have been demonstrated for specific conditions and populations. Behavioural and cognitive-behavioural treatments have more favourable outcomes than brief supportive treatments for anxiety disorders and for many conduct problems; family-based and multisystemic approaches outperform individual treatment for antisocial behaviour and for eating disorders; and trauma-focused treatments outperform nonspecific treatment for post-traumatic stress symptoms. These differences matter for theory, because they indicate that the mechanisms proposed by particular theories are operative in identifiable conditions.

**The literature on the limits of the comparison.** At the same time, the methodological quality of much of this literature is low by the standards of evidence-based medicine, and the strongest claim the evidence supports is that certain approaches, applied competently to well-defined problems in appropriately selected populations, produce better outcomes than alternatives that are less well matched to the problem. This is a claim about fit, not about a universal hierarchy of theories.

**Practice-based evidence.** Routine outcome monitoring in real services has repeatedly shown substantial variability in outcome between providers working in the same model with the same protocol, which is evidence for the importance of practitioner skill, relationship quality, and attention to the specific characteristics of the person. It also shows that outcome improves when practitioners receive systematic feedback, which is the strongest available argument for the integration of measurement into routine practice.

## 13. Strengths and Limitations of a Theory-Directed Practice

**Strengths.** A theory-directed practice is coherent, teachable, auditable, and generative. It supports supervision, because a supervisor and a supervisee who share a framework can examine a case rather than merely exchange impressions. It supports research, because a coherent model can be manualised and tested.

It supports professional identity and public accountability. And it supports the practitioner in the moment of difficulty, because a framework reduces the number of decisions that must be made from scratch.

**Limitations.** Theory can narrow attention, and a practitioner who has been thoroughly trained in one tradition can fail to see phenomena that another tradition foregrounds. Theory can be misapplied in the name of expertise. Theory can ossify, and a student who learned Rogers in 1965 may be using a version of person-centred practice that Rogers himself would not recognise. Theory can be used to protect the practitioner from the anxiety of not knowing, by providing a certainty that the situation does not warrant. And theory in the hands of an unskilled practitioner can be an instrument of considerable harm: a confident application of a poorly suited model to a young person is worse than an uncertain, attentive, unstructured approach.

None of these limitations is an argument against theory. Each is an argument for the disciplined, provisional, self-correcting commitment described in Section 9.

## 14. Cultural and Ethical Considerations

Two cultural points require statement at the outset of a graduate course, because they condition everything that follows.

First, the theoretical traditions surveyed in this book were developed predominantly in the United States, predominantly within a Euro-American cultural frame, and predominantly with adult clients. The conceptual apparatus of individual pathology, the primacy of the individual as the unit of intervention, the emphasis on verbal self-expression, the reliance on insight, and the assumption that the therapeutic relationship is a dyad are all cultural products rather than cultural universals. Individualism, the nuclear family as the normative family structure, the culturally specific norms of emotional display, the prioritisation of future orientation over present obligation, and the high value placed on self-authorship are additional examples. Chapter 8 develops this argument systematically, and the student should read it before reading any of the theory chapters as though the theory were universal.

Second, the ethical codes of the profession require cultural responsiveness as a matter of obligation rather than of optional excellence, and they define it in terms of the practitioner's self-examination: the codes require the counsellor to examine their own assumptions and biases, to understand the cultural context of the client, and to guard against the imposition of the counsellor's own values. The examination is not a one-off training exercise. Chapter 48 provides a model of the self-examination in the form of an analysis of power, and Chapter 49 applies it to a population for whom the harm of misreading is most severe.

## 15. Key Terms

**Key Terms at a Glance**

- **Theory** — A systematic, coherent, and testable set of propositions about persons, psychological difficulty, and the processes of change; in counseling, the organising framework of professional practice.
- **Model** — A simplified visual representation of the central features of a theory, usable without accepting every proposition of the theory from which it derives.
- **Framework** — An organising structure for practice that draws on several theories and specifies their relationships.
- **Technique** — A specific, replicable procedure for doing something in the counseling room, ordinarily embedded in a theory.
- **Protocol** — A manualised sequence of procedures with specified timing, order, and fidelity requirements, designed for replication.
- **Formulation** — A theory-laden account of the processes that maintain a young person's difficulties, written so that it can be tested and revised.
- **Mechanism of change** — The process by which an intervention is supposed to produce improvement; the causal claim that distinguishes a treatment from a procedure.
- **Common factors** — The elements shared across all recognised therapies, principally the therapeutic alliance, empathy, goal agreement, and expectancy.
- **Technical eclecticism** — The selection of techniques from any available tradition on the basis of fit, without requiring a unified theoretical account of their operation.
- **Theoretical integration** — The use of more than one theory together, with an explicit account of their agreements, conflicts, and the resulting coherent mechanism of change.
- **Pluralism** — The professional acknowledgement that different theories suit different people and problems, with the choice made on the case rather than on allegiance.
- **Dogmatism** — The treatment of a theory as a revelation rather than a hypothesis, with the consequent inability to hear disconfirming evidence.
- **Fidelity** — The degree to which an intervention is delivered as specified in the theory or protocol from which it derives.
- **Committed openness** — The professional stance of holding a theory strongly enough to act and weakly enough to be disconfirmed.
- **Telling** — The technique of commenting on the therapeutic process in order to elicit information from it.

**Key Terms at a Glance (continued)**

- **Present-centred therapy** — A stance that attends to and works with the immediate moment of the counseling session rather than with historical material.
- **Common factors model** — The account of therapy that locates the primary determinants of outcome in the nonspecific elements shared by all recognised therapies.
- **Treatment manual** — A document specifying the rationale, sequence, timing, and fidelity criteria of a treatment.
- **Case conceptualisation** — See *formulation*; the two terms are used interchangeably in the contemporary literature, with the British spelling increasingly preferred in the chapters of this book.

## 16. Chapter Summary

Theory in counseling is a systematic and testable account of persons, difficulty, and change. It performs four functions that no other professional resource can perform: it explains, it directs attention and sequences, it predicts, and it supplies the criteria by which the work is evaluated. These functions are converted into practice through a clinical loop in which observation leads to formulation, formulation to prediction, prediction to intervention, and intervention to evaluation, with revision at every step.

Theory is also ethically necessary. The ethics of informed consent, competence, the avoidance of harm, justice, and public accountability all presuppose a practitioner who can state what he or she believes is happening and why, and can show the basis of that belief. The absence of an articulated theoretical position is not freedom from constraint; it is constraint without examination.

The two failure modes of theoretical engagement are dogmatism, which forecloses correction, and nihilism, which mistakes the importance of the relationship for the dispensability of method. The defensible position is committed openness: a strong theoretical commitment held provisionally, with named conditions for revision.

The theory chosen has consequences. In the case of Nadia, a fourteen-year-old who has withdrawn from school, the behavioural, cognitive, humanistic, systemic, and trauma-informed readings each propose a different first action, and each first action changes the trajectory of the following weeks. That is the measure of theory's importance, and it is why the subject is worth a full course rather than a week of orientation.

## 17. Review and Discussion Questions

**Review and Discussion Questions**

1. Define a theory of counseling in your own words, and state what the definition excludes. What would be lost if the phrase "theory" were used to refer to any belief about human behaviour?
2. A colleague tells you that she has been trained in one theory and sees no need to look at the others because "the research shows the common factors are what matter." Identify at least three errors in her position.
3. Explain the difference between a technique and a mechanism of change, and give an example of a technique that has been used outside the theory that generated it. What is the risk of that transfer?
4. Using the Nadia case, specify what a behavioural and what a systemic practitioner would each do in the first hour, and state what evidence would distinguish which of them was correct.
5. Why is it legitimate to say that a formulation should be written so that it can fail? What goes wrong, in the case of a young person, when a formulation cannot be disconfirmed?
6. Distinguish technical eclecticism from theoretical integration, and give an example of a practice that is clearly one and clearly the other.
7. The chapter argues that the absence of a theoretical position is an ethical problem. Make the case against that argument as strongly as you can, and then state your own conclusion.
8. What would you include in the element of a personal theoretical position that you find hardest to write, and why?
9. Identify one widely used intervention in counseling with young people for which the evidence base is weaker than its popularity would suggest. How would a theory-committed practitioner respond to that finding?
10. How does the requirement that a theory be culturally responsive change the definition of theory given in Section 1?

## 18. Practice Activities

**Practice and Application**

### Activity 1.1 — The Five-Column Theory Notebook (2 hours, individual, then paired)

Draw five columns on a page: *assumptions about human nature*; *how difficulty develops*; *how difficulty is maintained*; *mechanism of change*; *techniques that follow*. For each of the following, complete the row: cognitive theory, behavioural theory, person-centered theory, family systems theory, and trauma-informed practice. Do not consult the chapters of this book; use your prior knowledge and mark the cells you are unsure of. Then compare with a partner, marking in a second colour the cells where you disagreed. Keep this notebook: it is the primary revision instrument for the course.

### Activity 1.2 — The Same Case, Five Theories (90 minutes, groups of four to five)

Assign each group a theoretical position from the Nadia case. Each group has forty minutes to prepare a five-minute briefing answering: what is happening with Nadia, what should happen in the next seventy-two hours, and what in your formulation could be wrong. In the second half hour, the groups hear each other's briefings without interruption and then identify the single point on which the competing positions most sharply disagree. Record that point; it is the raw material of the integrative argument in Chapter 51.

### Activity 1.3 — Writing Your Own Theoretical Position (3 hours, individual, submitted)

Complete the seven elements of Section 10 in no more than 1,500 words. Expect to find elements 3, 6, and 7 substantially harder than the others. In particular, do not submit a position whose element 7 you cannot write; the seminar will ask.

### Activity 1.4 — The Disconfirming Evidence Exercise (60 minutes, pairs)

Take a position that you personally hold about adolescent behaviour — about what causes school refusal, about what makes therapy work, about what a child needs from a parent. Write, in one paragraph, the observation that would most damage your position, and identify where you would look for it. Then discuss with a partner whether you would actually act on the finding if you encountered it. This is a small exercise about intellectual honesty, and it is deliberately uncomfortable.

## 19. Annotated Reading Pathways

This chapter's central claim — that theory is necessary, that it must be held provisionally, and that the commitment must be articulable — is best pursued through a small number of well-chosen works rather than through breadth. The following four pathways are suggested.

### Pathway A: The nature of theory in counseling

Begin with Tatum (2017) for a clear and compressed comparison of the major orientations, which will give you a map to hold in mind while reading anything else. Read Pope and Vasquez (2021) selectively: the opening chapters on the professional context and the closing chapters on integration are worth more than the middle. Then read the two chapters in this book that you find most difficult, and ask why you find them difficult.

### Pathway B: Common factors, the alliance, and the limits of technique

Frank (2015) remains the most thorough scholarly treatment of the common factors question and of the historical development of the therapeutic relationship; it is long, and the chapters on the placebo literature and on the treatment of the relationship as a necessary condition are the ones to prioritise. Read Wampold (2015) immediately afterwards, as a direct, short, and somewhat argumentative counterweight, and then read the chapter on the therapeutic relationship in Part I of this book. Finish by reading the meta-analytic reviews of the alliance-outcome relationship in adolescents that your library holds; the adolescent literature is thinner than the adult literature and is worth seeking out.

### Pathway B continued: Practice-based evidence

For the argument that outcomes depend on practitioner skill and on attention to the individual, read the primary practice-based research literature rather than the summaries. This strand of the literature grows more persuasive the more of it one reads, and it is the empirical foundation for the emphasis in this book on formulation, on measurement, and on the deliberate development of relational skill.

### Pathway C: Culture and theory

Read Chapter 8 in this book as an introduction, and then read the chapters on culture in the standard survey texts alongside it. Where the survey texts and this book disagree, read the primary sources cited in the notes. Pay particular attention to the material on the cultural embeddedness of the concept of the individual and on the limits of the dyadic therapy model.

### Pathway D: The integrative stance

Read Chapter 51 of this book, then read the integrative chapters of the standard survey texts, and then read a small number of integrative case formulations written by practitioners who show their reasoning. The most useful single exercise is to take one case, write a formulation from three different theoretical positions, and identify what each position sees that the others miss. That exercise is the reason the course is worth the effort, and Chapter 52 sets it up in detail.

## Practice Laboratory and Extended Worked Example

This laboratory turns **The Role of Theory in Professional Counseling Practice** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is why theory matters, what it does and does not do, and how it becomes a working instrument in the counseling room. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner

uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary

that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Amara is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Amara's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Amara explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Amara chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Amara has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Amara can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults

may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for The Role of Theory in Professional Counseling Practice

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **The Role of Theory in Professional Counseling Practice** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **The Role of Theory in Professional Counseling Practice** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

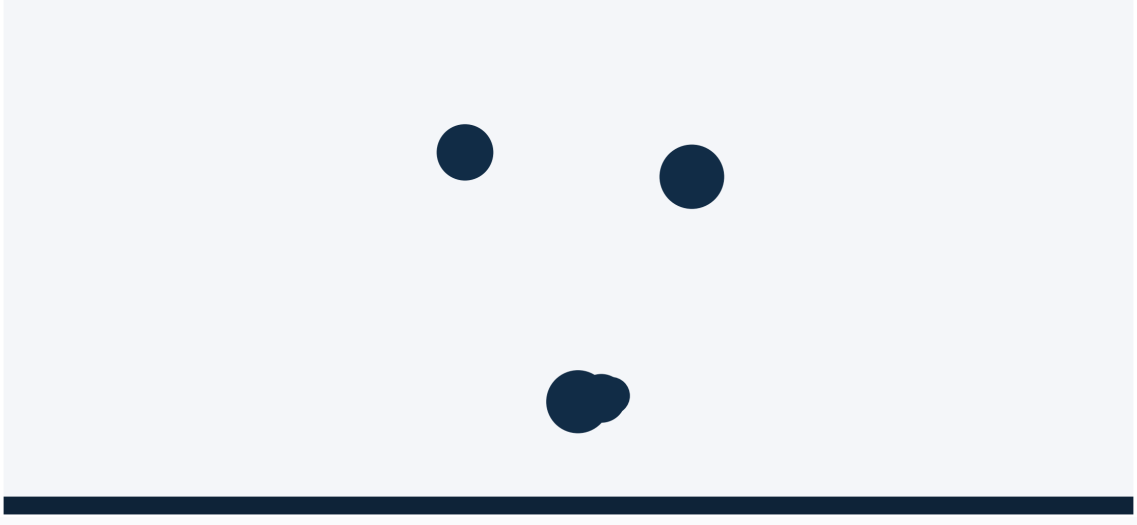
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **The Role of Theory in Professional Counseling Practice**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain The Role of Theory in Professional Counseling Practice to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 2

# Scientific and Professional Foundations of Counseling

The research tradition, the professional identity, and the movements that established counseling as a distinct discipline.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the social, medical, and educational conditions that gave rise to the counseling movement in the twentieth century.
- Identify the four intellectual traditions inherited from psychology and explain what each contributes to the counseling knowledge base.
- Distinguish the scientific, the professional, the ethical, and the practical dimensions of the counseling enterprise, and explain how they depend on one another.
- Describe the principal research methods used to evaluate counseling interventions and the questions each is able to answer.
- Explain what professional accreditation and licensure do and do not accomplish, and why the distinction between them matters ethically.
- Locate counseling within the broader landscape of mental health service provision and describe the domains in which counselors practise.
- Discuss the critique that counseling is a "semi-professional" occupation and evaluate its force.

## Chapter Overview

Counseling did not begin as a science. It began as a set of social responses to a set of social conditions: mass urbanisation, the large-scale entry of women into the industrial workforce, the recognition that many people who had been confined in asylums and hospitals for want of other resources could be treated in the community, the expansion of public education, and two devastating wars that left behind large numbers of people who were injured, bereaved, and confused about what they were for. The profession that emerged from these conditions was created by people who wanted to do something useful and who needed to be able to justify what they were doing to institutions that funded them.

That origin explains several persistent features of the profession, and it explains them better than an account that begins with Rogers or with the publication of a textbook. It explains why counseling has always been unusually concerned with the demonstration of effectiveness. It explains why the profession has always been internally divided between a scientific and a humanistic wing. It explains why the training of counselors has always included supervised practice rather than only academic study, and why the professional codes have always placed unusual weight on competence and relationship. And it explains why the profession's central theoretical question is not "what is true?" but "what is defensible in the presence of a person who needs help now?"

This chapter traces that history in outline, identifies the four intellectual traditions the profession inherited, describes the architecture of the professional enterprise as it now stands, and examines the research methods by which the profession holds itself accountable. It closes with a discussion of the status of the occupation and of the critique that has been levelled at it.

## 1. From Asylum to Profession: The Social Origins

### 1.1 The nineteenth-century background

The counseling movement has antecedents that run through the nineteenth century, and a student who knows only the 1940s will misunderstand the profession's character.

The first antecedent is the **asylum reform movement**. Throughout the early nineteenth century, the institutional response to mental illness in Britain and the United States was custodial: large, overcrowded asylums whose purpose was detention and whose economies depended on the cheapness of the conditions in which the patients were kept. The asylum reform movement, associated with the physician John Conolly in Britain and with Dorothea Dix in the United States, argued instead for **moral treatment** — the provision of a clean, orderly, humane environment on the supposition that many patients were suffering from conditions responsive to care, kindness, routine, and purposeful activity. Moral treatment did not cure much, and it was destroyed by the overcrowding that followed the collapse of its institutional financing, but the claim it embedded in the professional imagination was decisive: *the conditions in which a person is treated matter, and the treatment of the mind is a specialised human enterprise rather than a custodial routine*.

The second antecedent is the growth of **public education** and, with it, the professionalisation of teaching. The nineteenth century saw the creation of graded school systems, the establishment of teacher training institutions, and the introduction of the idea that the systematic education of children required special knowledge and special preparation. The school guidance movement that began in the United States in the 1890s — combining the work of the developmental psychologist James, the educational statistician Engel, and, decisively, the work of **Leta Stetter Hollingworth** — located the beginning of professional school counseling in the scientific study of individual differences and in the conviction that the school could be a setting in which individual development was systematically supported. The school counselor was thus the first of the counseling roles to be defined, and it remains the role with the clearest institutional base.

The third antecedent is the **vocational guidance movement**, which arose in the first decade of the twentieth century and which was, in effect, the first organised attempt to apply psychology to the practical problems of human lives. Frank Parsons' work in Boston is conventionally taken as the beginning of vocational guidance, and Parsons' threefold formulation of matching, on the basis of the study of the individual, the occupations that the individual might fit, is the direct ancestor of the person-environment fit tradition that still organises career counseling.

The fourth antecedent is the **child guidance movement**, which began in the United States in the 1880s with the establishment of the first psychological clinics and which, by the 1930s, had produced the influential child guidance clinics. These clinics brought together a child, a family, and a set of professionals, and they developed the practice of working with a child's difficulties in the context of the family and the school rather than in the clinic alone. The contemporary practice of family assessment and school consultation descends directly from them.

## 1.2 The two world wars

The First World War produced the first large-scale demonstration that the systematic study of personality could be put to practical use. The military authorities required rapid assessment of recruits, and the development of the Army Alpha and Army Beta tests in 1917 represented a decisive advance in applied psychometrics. It also produced the first mass psychological casualty, the phenomenon subsequently called shell shock and now understood as combat-related post-traumatic stress. The observation that a proportion of soldiers could not return to civilian life, and that these men were not malingering but permanently altered, established both the clinical reality of trauma and the institutional necessity of specialised provision.

The Second World War was more consequential still, and its consequences were structural rather than symbolic. Three developments in particular shaped the post-war counseling profession.

The first was the **mass expansion of clinical psychology and psychiatry**, driven by the needs of returning servicemen and by the Veterans Administration hospitals, the largest psychological care system ever created. The evaluation and treatment programmes of the 1940s created a demand for trained practitioners, and the training programmes that met that demand produced the first generation of academically credentialed counselors and psychologists.

The second was the emergence of the **child guidance clinic as the centre of professional practice**, and with it the disciplinary formation of clinical child psychology. The influential Chicago and Boston clinics of the 1930s and 1940s, and the publications that emerged from them, established the model of a child clinician working with a family and with a school.

The third was the **rise of the mental health movement as a public policy concern**, and in particular the establishment in the United States of the National Institute of Mental Health in 1949, with its mandate for research, training, and service development. The creation of a federal research-and-training infrastructure is what made it possible for a profession to assert that it was scientific.

### 1.3 The four conditions of professional formation

Drawing these threads together, counseling became a profession in the middle of the twentieth century because four conditions were met.

| Condition | What it provided | Institution that embodied it |
|---|---|---|
| A distinct area of practice | A domain of work distinguishable from psychiatry, psychology, and social work | The school counselor, the rehabilitation counsellor, the marriage counsellor |
| A body of knowledge | A recognised literature and a shared curriculum | University departments and professional accreditation |
| A method of training | A standard programme combining academic study with supervised practice | Graduate programmes and practicum training |
| A professional identity and code | A shared set of commitments, standards, and ethical obligations | Professional associations and codes of ethics |

Each of these is necessary; none is sufficient alone. The profession's characteristic tensions — between the humanistic and scientific wings, between the school and clinical settings, between the academic and the vocational routes to qualification — are products of the fact that the four conditions were met at different times, in different institutions, for different populations.

## 2. The Four Intellectual Traditions

Counseling inherited four distinct intellectual traditions from psychology, and its internal structure reflects their coexistence rather than the triumph of any one of them.

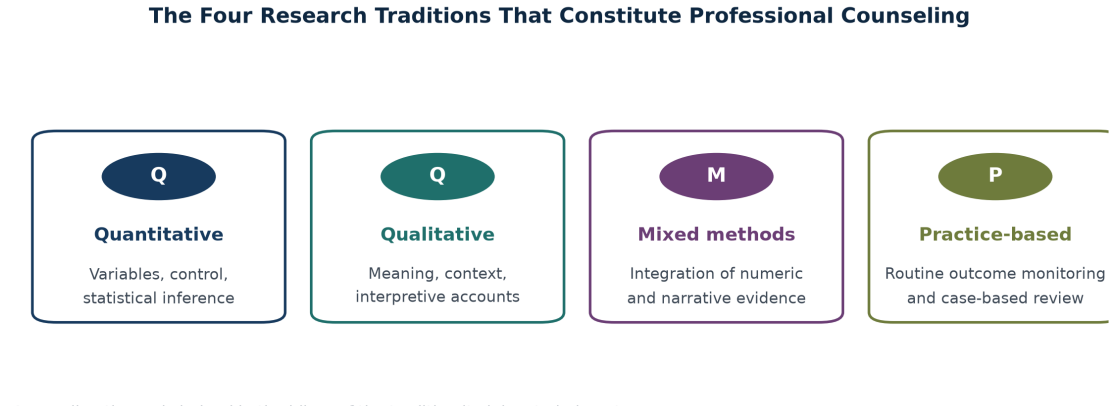



**Figure 2.5** The four research traditions that constitute professional counseling.

## 2.1 The experimental tradition

The experimental tradition begins with the founding of the laboratory and reaches its twentieth-century culmination in behaviourism. Its contribution to counseling is the **methodological** contribution: the experimental design, the operational definition, the single-case experimental method, the randomised controlled trial, and the general insistence that claims about what helps must be testable. The behaviourist wing of counseling is the direct inheritor of this tradition, and the entire enterprise of treatment manuals, efficacy studies, and fidelity monitoring in the 1980s and 1990s was a product of the experimental tradition applied to a clinical field.

## 2.2 The interpretive tradition

The interpretive tradition descends from the hermeneutic and phenomenological philosophies and entered psychology through the psychodynamic movement, the humanistic movement, and the later qualitative research tradition. Its contribution to counseling is the **understanding of meaning**: that behaviour has intelligibility, that symptoms have communicative and adaptive functions, and that the practitioner's task is first to comprehend and only then to change. The tradition's methodological contribution, qualitative interviewing and the analysis of meaning, entered counseling research more slowly but is now indispensable to the study of process, of the alliance, and of the client's own account of what has helped.

## 2.3 The humanistic and developmental tradition

The humanistic and developmental tradition, given its decisive expression in Abraham Maslow's work on motivation, in the research on self-actualisation, and in Kurt Lewin's field theory and group dynamics, contributed the concept of the **person as a self-actualising, socially embedded agent**. It supplied the concept of the fully functioning person, the person as the expert on his or her own life, the concepts of congruence and unconditional positive regard, and the group work tradition. It also contributed the *person-environment fit* framework of vocational psychology.

## 2.4 The systems and sociocultural tradition

The systems tradition, from general systems theory, cybernetics, and ecological psychology, contributed the concepts of **wholeness, circularity, feedback, and context**. It supplied the family systems and ecological frameworks that occupy Part V and Part VII of this book, and it is the intellectual source of the contemporary emphasis on context, on the interaction of person and environment, and on multilevel intervention.

### 2.5 The persistence of the division

The coexistence of these traditions explains the two great splits in the history of counseling: the split between the humanistic and the behavioural wings that dominated the 1950s and 1960s, and the split between the "training model" and the "clinical model" within graduate preparation. Neither split was a failure of goodwill. It reflected genuine and, within limits, irreconcilable differences about the nature of the person — whether the person is an active chooser who can be trusted, or a system of responses to contingencies that must be rearranged. Chapter 3 examines that question directly, because the question of what a person is remains the foundation of everything else.

## 3. The Architecture of the Professional Enterprise

Counseling today is a large and heterogeneous enterprise. It is worth describing its parts precisely, because students of the profession often meet several of them in a single week and need to know the differences.

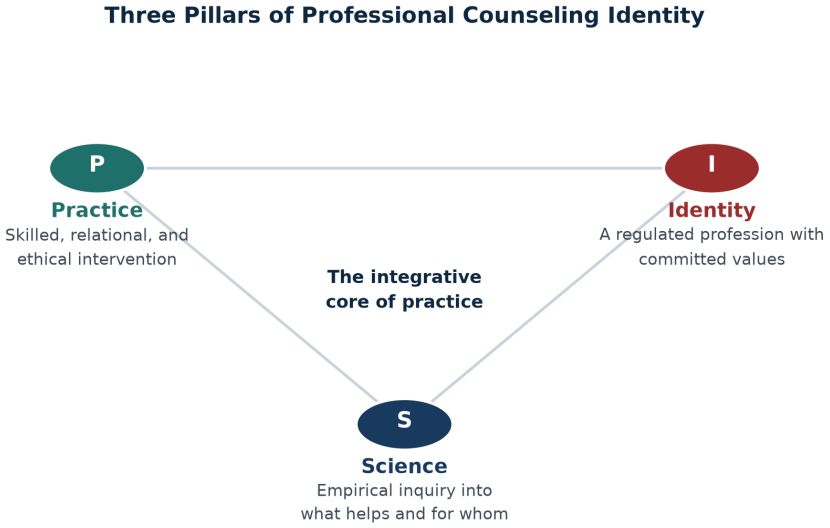



**Figure 2.6** Three pillars of professional counseling identity.

### 3.1 The domains of practice

**School counseling.** The school counselor works within an educational institution, with a caseload that typically includes individual students, small groups, whole-class programmes, consultation with teachers, and liaison with families and community agencies. The distinctive constraints of the role are institutional: the school day, the school calendar, the school building, the collective responsibility of the staff, and the fact that the client is usually not the person who sought help. Chapter 56 treats school practice in detail.

**Clinical and counseling practice in community mental health.** The community clinician works in a health centre, a hospital outpatient department, a private practice, a youth centre, or an agency, and typically sees a more selected population with more severe difficulties. The distinctive constraints are the reimbursement framework, the necessity of diagnosis for service eligibility, and the requirement of outcome reporting.

**Substance use and mental health services.** The substance use field, which is in some respects the most theoretically developed part of the profession, operates under a specific regulatory framework, and the treatment of co-occurring substance use and mental health difficulties is the field's characteristic technical problem. Chapter 14 in the curriculum map addresses it.

**Marriage, couple, and family therapy.** Family practice treats the relationship as the unit of intervention, and its distinctive technical problem is that the client who is referred is frequently not the person who is most distressed. Chapter 33 addresses work with parents and adolescents.

**Career and vocational counseling.** Vocational practice is organised around person-environment fit and is strongly shaped by the labour market and by the institutions of education and training. It retains the clearest link to the movement's nineteenth-century origins.

**Rehabilitation, disability, and developmental services.** Practising with people experiencing disability requires a theoretical position that does not locate disability in the individual, and this requirement has produced some of the most theoretically sophisticated work in the profession.

### 3.2 Settings and the shape of the work

Whatever the domain, the conditions of work are common across settings and worth naming, because they shape what is professionally possible. A counselor works within a setting that has its own goals, its own accountability structures, and its own informal culture. A counselor works within a legal and regulatory framework that determines what may be done, with whom, and with what record. A counselor works within an organisational structure that determines the pace of the work, the caseload, the referral pathways, and the extent to which collaboration is possible. A counselor works with a client who is usually in a setting the client did not choose. These conditions are not distractions from the counselling; they are the field in which the counselling occurs, and the professional competence that Chapter 57 assesses includes the capacity to work competently within them.

## 4. The Research Foundation

The professional claim to scientific status rests on a body of method, and a graduate student should be able to name and describe the principal methods, because the reading of the research literature in Part VII of this book requires it.

### 4.1 The randomised controlled trial

The randomised controlled trial assigns participants to conditions, ideally with allocation concealed and with assessors blind to condition, and compares outcomes. It is the strongest design for causal inference and the standard against which other designs are judged. Its limits in counseling research are severe: the sample sizes required for adequate power are large, so trials are usually conducted with adults rather than with young people; the interventions tested are usually manualised, which changes what is being tested; and the

trials are frequently conducted in research clinics that do not resemble the services in which counselling is ordinarily delivered.

## 4.2 The randomised controlled trial in routine services

The most methodologically important innovation in the field has been the randomised controlled trial conducted within routine services with the outcome data of ordinary clients. This design, often described as a pragmatic or cohort-randomised design, has produced a very different and more sobering picture of counselling effectiveness than the efficacy literature. In large trials in which routine service providers delivered the treatments under routine conditions, average outcomes were considerably less favourable than in the efficacy literature, and the variability between providers was substantial. The implication is not that counselling does not work. It is that its effectiveness depends on conditions that the efficacy literature had abstracted away: the skill of the practitioner, the quality of the relationship, the fit between the intervention and the person, and the quality of the service system in which the work is done. Chapter 7 discusses this literature in detail, and Chapter 53 draws out its implications for measurement-based practice.

## 4.3 Single-case experimental designs

The single-case experimental design tracks an individual's outcome over time, with repeated measurement and, typically, the systematic manipulation of one variable. Its central weakness is generalisation to other individuals, but its strength is that it can be conducted within a routine service, that it provides immediate feedback to the practitioner, and that it can capture the individual young person whose case is at issue. Behavioural approaches have historically been the most willing to use these designs, and they remain a legitimate and underused option in school and agency settings.

## 4.4 Meta-analysis and systematic review

Meta-analysis pools the results of multiple studies, weighting each by its precision, and estimates an overall effect with a confidence interval. Systematic review differs in that it is a protocol-governed search and appraisal of a defined literature with a pre-specified inclusion criterion. Both are essential to the appraisal of the evidence, and both have to be read critically: the quality of a meta-analysis depends heavily on the quality of the primary studies, the homogeneity of the interventions, and the risk of publication bias in the source literature, which is substantial in psychotherapy research and is estimated in many reviews to run into the tens of percent.

## 4.5 Qualitative and mixed-methods research

Qualitative research asks what the intervention means, how it is experienced, and what happens in practice that does not appear in outcome data. It is the method by which the process of therapy has been understood, and it is indispensable to the study of the alliance, of client experience, and of the reasons why a manualised treatment is delivered differently in practice than in a trial. Mixed-methods research integrates the two, and its quality depends on the integration being specified in advance rather than improvised at the writing stage.

## 4.6 Practice-based evidence

Practice-based evidence is generated by routine outcome monitoring in real services, aggregated across caseloads, and analysed with quasi-experimental methods. It is the only body of evidence that describes

what happens in ordinary services with ordinary clients, and it is the most direct challenge to the assumption that a treatment proven in a trial will work in a service. Chapter 53 develops the instruments.

## 5. Regulation, Accreditation, and Licensure

Three distinct mechanisms govern entry to and practice of the profession, and they are frequently confused.

**Accreditation** is a quality assurance mechanism applied to educational programmes rather than to individuals. In the United States, the Council for Accreditation of Counseling and Related Educational Programs accredits degree programmes against published standards that specify faculty qualifications, student-to-faculty ratios, curriculum content, field experience, and assessment practices. Accreditation is a statement about the programme, and it is a precondition of many forms of state licensure.

**Licensure** is a state-level legal requirement that an individual must hold a licence to practise. Its design varies considerably by jurisdiction, and the differences are not merely technical. In some states the licensure track is a two-year master's degree with a supervised practicum and a national examination; in others it is a doctoral degree with additional supervised hours. A student considering practice in a specific jurisdiction should establish the requirements early, because the implications for training and for the sequencing of coursework are substantial.

**Certification** and **registration** are alternatives used in some jurisdictions in place of licensure, with the usual consequence that the title of the professional is not legally protected.

The ethical significance of the distinction is that licensure is a public protection mechanism, and it is the mechanism through which the state's interest in the competence of those who treat its citizens is expressed. It is also the mechanism through which the profession has historically accepted a measure of state oversight in exchange for the protection of its own scope of practice. The profession's complicated relationship with licensure — the recurring disputes over the independent practice of counselling, the movement to create a distinct licensure for licensed professional counsellors, the "licensure compact" initiatives — is best understood as a struggle over the distribution of power between the state, the profession, and the other mental health occupations.

### 5.1 What the codes require

The ethical codes of the major professional bodies require a great deal, and a graduate student should read them closely, because they constitute the legal and moral floor of practice. Among their provisions are: the requirement of competence and of the maintenance of competence through continuing education; the requirement of informed consent, including, for minors, the obtaining of parental or guardian permission and the young person's assent; the requirement of confidentiality and the specification of its limits, including the duty to disclose when a client is a danger to self or others and the child-protection reporting duties that apply in most jurisdictions; the requirement of informed consent and of the limits of the confidentiality of group and family work; the requirement that the practitioner not discriminate on the basis of race, ethnicity, national origin, religion, sex, gender identity, sexual orientation, age, disability, or socioeconomic status; the requirement of appropriate boundaries and of the avoidance of dual relationships, particularly in work with minors; the requirement of competence in the use of assessment instruments and of the obligation to use instruments for their intended purposes; the requirement to use the most current and relevant evidence in practice; and the requirement to attempt to resolve ethical issues through an orderly process, beginning with consultation and supervision and escalating to regulatory bodies where necessary.

These requirements are the framework within which every theoretical decision in this book must be made.

## 6. The Critique of the Semi-Profession

Counseling has attracted a recurring and not wholly unfair criticism that it is a **semi-professional** occupation: a group with pretensions to professional status that lacks some of the hallmarks of a profession. The classic criteria of a profession are a distinctive body of knowledge, entry controlled by a formal educational process, autonomy in the conduct of practice, a code of ethics with enforcement, a commitment to a lifetime of continuing education, a social organisation with a formal structure, and the acceptance of a distinctive social responsibility.

Measured against these criteria, counseling does reasonably well on the body of knowledge, on entry, on ethics, and on continuing education, where many jurisdictions now require it. It does less well on autonomy, because much of counselling is delivered in settings in which the organisation determines what is done, to whom, and for how long. It does least well on the social organisation, because the profession has historically been divided among several competing bodies with different requirements, and on the question of a distinctive social responsibility, because the profession's answer to "what are you for?" has historically been ambiguous.

The most useful response to this critique is neither to dismiss it nor to accept it wholesale. Two of its claims require sustained response in the present circumstances. The first is the autonomy critique, which is the substance of the argument for the integration of the profession and for the recognition of counselling as a distinct mental health discipline rather than as a specialism of psychology or social work. The second is the equity critique, which is the substance of the argument that the profession's commitments to justice require attention to its own internal arrangements — who is admitted, who is trained, who is paid, and who is treated.

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## Further Reading for This Chapter

- Pope, S. J., & Vasquez, M. J. T. (Eds.). (2021). *Theories of counseling and psychotherapy* (6th ed.). Pearson. — Its opening chapter on the professional context is the best short account of the material in Sections 3 and 5.
- American Counseling Association. (2014). *ACA code of ethics*. — Read Sections 1 and 5 of this chapter alongside the code itself; the contrast between the code and the profession's self-description is instructive.
- American Psychological Association. (2017). *Ethical principles of psychologists and code of conduct*. — Note the differences in structure from the counseling code, and consider what they imply about the two professions' self-conceptions.
- American Psychiatric Association. (2013). *Diagnostic and statistical manual of mental disorders* (5th ed.). — Reference for the nosological framework referred to in the chapter; students should be able to locate the diagnostic criteria for the conditions they encounter in placement.
- Kutchins, H. I., & McHugh, A. (2002). *Genealogy and the history of psychology: Diverging views of the clinic and the laboratory*. — For the more advanced student, an account of the professional and

epistemic stakes of the laboratory–clinic divide.

## 7. Case Illustration: The Referral That Did Not Arrive

The following composite case is used in the course to examine the professional and organisational dimensions of practice — the dimensions that theory addresses only indirectly.

**The referral.** A sixteen-year-old boy, Tyron, is referred by his school attendance officer to the agency's school-based counseling service. The referral states: "Non-attendance increasing. 40% absence last term, 60% this term. Previously good. Behaviour in class disruptive when present. Family engaged. Please advise."

**The agency.** The agency is a community mental health provider with a contracted school-based service funded through a public health contract. The contract specifies that the service will provide brief counseling to referred students, up to eight sessions, with referral back to the school at the end of the episode. The contract does not specify the number of referrals the service is expected to accept, and the service currently carries a caseload of 94 students across four full-time equivalent staff, of whom two are vacancies being filled on a temporary basis. The waiting time for a new referral is currently three weeks.

**The practice.** The counselor making the allocation is a qualified professional counsellor with two years of post-qualification experience. She reviews the referral and notes that it contains no information about the family, no information about the pattern of absence, and no information about what happens at home or at school on the days Tyron does attend. She has a fifteen-minute allocation meeting the following morning and one other referral to consider, a thirteen-year-old girl with a suspected eating disorder whose referral is marked urgent.

**The constraints.** Tyron's school is considering a formal attendance process with a view to a referral to the local authority's attendance and behaviour support team, which would bring the family's benefits into play and, in the view of the school, would be "the thing that will actually get them to engage." The school's designated safeguarding lead has, in a previous case, suggested that "the family just need to be told properly."

## 7.1 What the professional foundations require of the counsellor

**On the ground of allocation.** The referral is under-specified, and the ethical question is whether the counsellor can accept it as it stands. The codes require that the counsellor work within the limits of competence and that the client receive a service that is appropriate to need. A fifteen-minute allocation meeting that triages a referral on the basis of a one-paragraph administrative summary is a decision with substantial consequences, and the code requirement of informed consent implies, at minimum, a requirement that the young person be informed of what has been decided about them and by whom.

The defensible action is not to refuse the referral, which would leave Tyron without service and would reproduce the school's expectation that non-engagement is a family failure. The defensible action is to accept the referral while specifying what will be done: a brief telephone contact with the school to obtain the attendance pattern and the two most recent incidents; a letter to the family requesting a meeting that states the service's role plainly and does not imply a school disciplinary process; and a planned review point at the

end of the first two sessions to consider whether the brief model is appropriate. This is a routine and unremarkable piece of professional judgement, and it is a small illustration of the general proposition that professional competence consists in the routine management of such details.

**On the pressure from the school.** The school has proposed attendance enforcement as the route to engagement. Two ethical and clinical questions arise. The first is whether the school is right: research on compulsory school attendance procedures shows mixed effects, with the strongest support for approaches that include a functional assessment of the reasons for absence and a family-based intervention, and considerably weaker support for purely punitive approaches. The second question is what the counselor's role is. The counselor is employed by an agency, not by the school, and owes duties to the client that are not subordinate to the school's administrative convenience. Declining to participate in a disciplinary process where the counselor believes it will be harmful to the young person is a legitimate professional act, provided it is done transparently and with reasons given, and provided the alternative is clear. The counselor's letter to the school should therefore state, in plain terms, what the service will offer, what the service will not do, and why.

**On the competing priority.** The thirteen-year-old with a suspected eating disorder and the sixteen-year-old with a non-attendance pattern are not comparable cases. A 60% absence rate is serious but is not, on the information available, acute; a suspected eating disorder in a thirteen-year-old may be, and the case has been marked urgent by a professional with access to information the allocator does not have. Prioritising accordingly is correct, and the ethical failure would be to let the perceived urgency of the school referral displace the clinically indicated one. It is worth noting that the three-week waiting time is itself an ethical problem, and that it is a problem of service design rather than of individual professional conduct. Chapter 50 returns to the question of how protective factors and systemic responsibility are distributed.

**On the limits of the brief model.** The contract specifies eight sessions. The research on brief treatment is clear that brief work can be effective, but it is effective when the intervention is focused, when a functional formulation has been made, and when the work is designed to end. Eight sessions allocated to a case with no assessment information and a strong suggestion that the problem is oppositional or defiant is unlikely to be effective, and the risk is that the episode is closed and the referral back to the school reproduces the original difficulty in a worse form. The ethical move is to make the brief model's limits explicit at the point of allocation rather than at the point of discharge.

## 8. Evidence and Research Support

The claim that counseling is a research-based profession is defensible, but it requires precision about what the research base actually supports.

**What is well supported.** That a substantial proportion of young people who receive well-delivered, theory-based counseling improve, and that the improvement is measurable by independent assessment as well as by self-report. That specific treatments, competently delivered, produce superior outcomes to nonspecific attention in well-defined conditions, particularly for anxiety and for conduct problems. That the quality of the therapeutic relationship is a consistent and substantial predictor of outcome across therapies and ages. That practitioner competence, the quality of the alliance, and the fit of the intervention to the person each contribute to outcome beyond the contributions of the treatment protocol.

**What is poorly supported.** That any one theory is superior to any other in general; the evidence does not support a general hierarchy. That a manualised protocol delivered by a competent practitioner in a

well-resourced service is the best available treatment for most young people; the pragmatic trial literature suggests otherwise. That outcome can be predicted accurately from diagnosis; the literature on diagnosis and outcome is a literature of modest prognostic power. That the profession's confidence in its own evidence base is well calibrated; systematic reviews of counseling research consistently report a substantial risk of bias in the primary literature and a significant risk of publication bias.

**What is methodologically weak.** The dominant methodological weakness of the counseling research literature is the overwhelming reliance on samples of adults in outpatient treatment. The evidence base for the treatment of children and adolescents is thinner, more heterogeneous, and more likely to rely on uncontrolled designs than is the evidence base for adults. This is a serious problem for a profession whose declared area of practice is children and adolescents, and it is one reason why the developmental adaptations discussed throughout this book are not simply matters of clinical taste. They are matters of evidentiary honesty.

**What follows for practice.** The evidence supports a disciplined, responsive, monitored, and competent practice; it does not support the confident application of a protocol to a case that has not been assessed, nor the assumption that a young person's diagnosis determines the treatment. It also supports the profession's current emphasis on measurement, on feedback, and on the routine evaluation of outcomes as a condition of ethical practice.

## 9. Strengths and Limitations of the Professional Enterprise

**Strengths.** The profession has a substantial and growing research base, a clear ethical infrastructure, a recognised training standard, an expanding scope of practice, and an unusual breadth of settings in which it works. Its commitment to supervised practice and to relational skill is a genuine professional strength and one that is not shared by every adjacent occupation. Its engagement with school settings places it close to the ordinary conditions of young people's lives, which is where most difficulties are actually produced and maintained.

**Limitations.** The profession is divided among competing bodies with different standards, and this fragmentation weakens both the public protection and the professional coherence. Its status as a "semi-profession" is not simply an external slur; it reflects genuine internal weaknesses, including the reliance on employers to provide the settings and the structures within which competence is developed, and the resulting dependence of the profession's ethical authority on organisations that are themselves subject to financial and political pressure. Its research base is thinner for its core population than for adjacent ones. Its demographic composition does not reflect the population it serves, and this is a documented and persistent problem in every jurisdiction in which it has been measured.

## 10. Key Terms

**Key Terms at a Glance**

- **Counseling movement** — The twentieth-century social movement that produced school counseling, vocational guidance, mental health education, and the professions of school, vocational, and clinical counselling.
- **Moral treatment** — The nineteenth-century reform doctrine that the conditions in which a person is treated materially affect their recovery; the founding moral claim of the counseling profession.
- **Child guidance clinic** — The early twentieth-century clinic in which the child, the family, and the school were assessed together; the origin of family assessment and school consultation practice.
- **Person-environment fit** — The vocational psychology framework, derived from Parsons and developed by Super and Holland, in which career satisfaction depends on the congruence between a person's characteristics and the demands and rewards of an environment.
- **Experimental tradition** — The laboratory-based tradition in psychology that contributed the randomised trial, the operational definition, and the single-case design to counseling.
- **Interpretive tradition** — The hermeneutic and phenomenological tradition that contributed the understanding of meaning, the analysis of process, and the qualitative research method.
- **Humanistic and developmental tradition** — The tradition of Maslow, Lewin, and Rogers that contributed the concept of the self-actualising, socially situated agent.
- **Systems tradition** — The general-systems and cybernetics tradition that contributed wholeness, circularity, feedback, and context to counseling theory and practice.
- **Efficacy trial** — A controlled study of a treatment under ideal conditions, which establishes whether an intervention *can* work.
- **Pragmatic trial** — A randomised study conducted within routine services, which establishes what an intervention achieves in ordinary conditions with ordinary clients.
- **Single-case experimental design** — Repeated measurement of one individual's outcome with systematic manipulation of one variable, permitting causal inference at the level of the individual case.
- **Meta-analysis** — The statistical pooling of results across studies, weighted by precision, to estimate an overall effect with a confidence interval.
- **Systematic review** — A protocol-governed search, appraisal, and synthesis of a defined body of literature against pre-specified inclusion criteria.
- **Publication bias** — The tendency of studies with positive or null results to remain unpublished, which inflates pooled effect estimates; substantial in psychotherapy research.

**Key Terms at a Glance (continued)**

- **Accreditation** — Quality assurance applied to educational programmes against published standards, distinct from the licensing of individual practitioners.
- **Licensure** — A jurisdiction's legal requirement that an individual hold a licence to practise; the mechanism through which the state protects the public.
- **Scope of practice** — The legally and professionally defined boundary of a discipline's authority, the boundaries of which are frequently contested between counseling, psychology, social work, and psychiatry.
- **Common factors** — See Chapter 1; the elements shared by all recognised therapies, the systematic study of which is the common factors tradition.
- **Practice-based evidence** — Evidence generated by routine outcome monitoring in real services and analysed with quasi-experimental methods.
- **Semi-professional** — A characterisation of occupations that meet some but not all of the criteria of a full profession, most often applied to counseling.

## 11. Chapter Summary

Counseling emerged in the early twentieth century from the convergence of asylum reform, public education, vocational guidance, child guidance, and the two world wars. It became a profession when four conditions were met: a distinct area of practice, a body of knowledge, a method of training, and a professional identity with a code of ethics.

The profession inherited four intellectual traditions — experimental, interpretive, humanistic-developmental, and systems — and its internal structure reflects their coexistence rather than the victory of any one. The experimental tradition supplied method; the interpretive tradition supplied the understanding of meaning; the humanistic tradition supplied the concept of the self-actualising, socially situated agent; the systems tradition supplied wholeness, feedback, and context.

Professional practice is now organised in several domains with distinct settings, constraints, and technical problems, and is governed by a combination of accreditation, licensure, and ethical codes. The research foundation is substantial but uneven: the evidence supports specific treatments for well-defined conditions, the consistency of the alliance–outcome relationship, and the contribution of practitioner competence, but it does not support a general ranking of theories, and the evidence base for children and adolescents is thinner than the profession's own rhetoric suggests.

The critique that counseling is a semi-profession is not wholly unfounded. Its two most serious components are the question of professional autonomy and the question of internal justice, and both are live issues in the present period.

## 12. Review and Discussion Questions

### Review and Discussion Questions

1. Identify the four conditions that made counseling a profession, and give one example of a contemporary occupation that meets three of them and fails the fourth. What would the consequences of the failure be?
2. Explain the moral treatment doctrine and identify three contemporary practices in counseling that are its direct descendants.
3. Compare the contributions of the experimental and the interpretive traditions to the counseling knowledge base. Which of the two questions in this chapter — "does it work?" and "what does it mean?" — is better served by each?
4. A student says, "The evidence shows that all therapies are equivalent, so it does not matter which theory you use." Write a two-paragraph response that is both accurate and practically useful.
5. What is the difference between accreditation and licensure, and why does the difference matter to a student choosing a training programme?
6. Describe the difference between an efficacy trial and a pragmatic trial, and explain what each of them would tell you about a proposed school-based programme for anxiety.
7. Identify one feature of the counseling profession's research base that you think should concern the profession, and suggest what could be done about it.
8. In the Tyron case, what are the ethical problems in the referral as it stands? What would you do in the first week, and what would you document?
9. What does the semi-professional critique identify correctly? Argue the case against it, then say how much of it you accept.

## 13. Practice Activities

### Practice and Application

### Activity 2.1 — Mapping the Professions (90 minutes, small groups)

Each group takes one adjacent profession (psychiatry, clinical psychology, marriage and family therapy, school social work, substance use counselling, occupational therapy) and produces a one-page comparison against the four conditions of professional formation, the ethical code, the licensure arrangements, the research base, and the settings of practice. The groups present in a fifteen-minute plenary. The purpose is not to rank the professions but to make the structural differences concrete enough that a student can locate their own professional identity within the mental health system.

### Activity 2.2 — Reading a Pragmatic Trial (2 hours, individual preparation, 1 hour seminar)

Obtain the full report of a large pragmatic or cohort-randomised trial of a common counselling intervention for adolescents, using your library's databases. Produce a one-page brief answering: what was the population, what was the comparator, what was measured, what was the primary result, what was the provider-level variability, and what would this result change about the service in which you plan to work? Bring the brief to seminar and compare it with the results others obtained from different trials.

### Activity 2.3 — The Referral Audit (75 minutes, individual)

Take a referral from a placement, a placement advertisement, or a hypothetical case. List every piece of information the referral contains, and every piece of information you would need in order to allocate it safely. Then write the two or three questions you would ask the referrer to close the largest gaps. Finally, write the single sentence you would say to the referrer to set expectations about what the service can and cannot do. This exercise is short, and it is the most immediately useful of the chapter's activities.

## 14. Annotated Reading Pathways

**For the history of the profession.** The standard survey texts in counseling theory open with a chapter on professional and historical context; the fullest available account is in the historical chapters of the encyclopedic handbooks, and the profession's own institutional histories are available from the founding bodies. Read the pre-1940 material in order, because the twentieth-century profession acquired its characteristic moral commitments from the nineteenth-century ones and does not always know it.

**For the ethics.** Read the ACA code of ethics and the APA code side by side, then read a substantial critical commentary on each. The commentary matters more than the codes, because the codes tell you what is required and the commentary tells you what is at stake. Pay particular attention to the sections on minors, on confidentiality and its limits, on boundaries, and on cultural responsiveness, since these generate most of the difficult cases in youth work.

**For the research foundation.** Read one good systematic review of a specific intervention with adolescents, and then read the corresponding meta-analysis, and then find the primary trials and read one in full. The three together will teach you more about the state of the evidence than any textbook chapter, including this one. Note the risk-of-bias assessments, and note where they differ between the review and the meta-analysis.

**For the critique of the profession.** Read the strongest available statement of the semi-profession critique and the strongest available response to it, in that order, with attention to what each takes for granted. Then read the profession's own contemporary self-descriptions and assess them against both.

# Practice Laboratory and Extended Worked Example

This laboratory turns **Scientific and Professional Foundations of Counseling** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the research tradition, the professional identity, and the movements that established counseling as a distinct discipline. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Jonas is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Jonas's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Jonas explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Jonas chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Jonas has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Jonas can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Scientific and Professional Foundations of Counseling

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Scientific and Professional Foundations of Counseling** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Scientific and Professional Foundations of Counseling** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

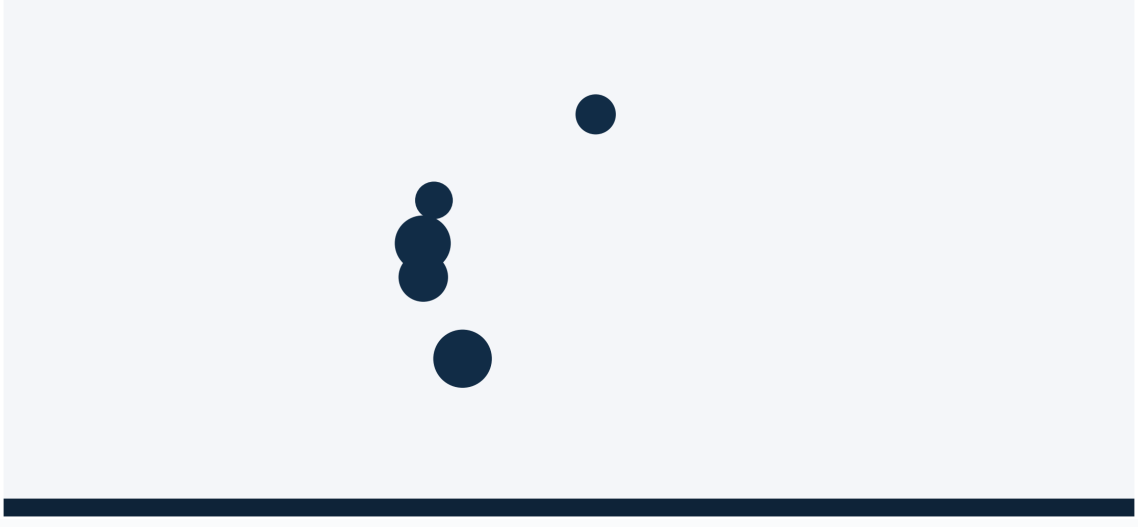
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Scientific and Professional Foundations of Counseling**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Scientific and Professional Foundations of Counseling to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 3

# Philosophical Worldviews: The Five Domains of Theory

Humanistic, cognitive, behavioral, systemic, and constructivist assumptions examined side by side.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain what a philosophical worldview is and why the foundations of a counseling theory are philosophical before they are technical.
- Describe the ontological and epistemological commitments of the humanistic, cognitive, behavioural, systemic, and constructivist domains.
- Identify the metaphysical, epistemological, and ethical commitments that distinguish each of the five domains.
- Explain why a theory's answer to "what is a person?" determines its answer to "what is wrong?" and "what should be done?"
- Apply the five-domain framework to a single clinical case and identify which domains illuminate and which obscure.
- Critically evaluate the five-domain taxonomy itself, including the cultural assumptions embedded in it.
- Distinguish theoretical domains from specific theories, and explain why the distinction matters in practice.

## Chapter Overview

Every theory of counseling is, beneath its technique, a philosophical position. A theory that assumes human beings are rational agents who choose in response to meaning will generate a different practice from one that assumes human beings are complex systems of learned responses to contingencies; and neither assumption can be derived from data, because both are frameworks for interpreting data. Chapter 1 argued that theory is necessary in professional practice. This chapter asks the prior question: what is theory made of, and what are its foundations?

The framework used here is the five-domain taxonomy that has organised the survey of counseling theory since the 1960s: the humanistic, cognitive, behavioural, systemic, and constructivist domains. The taxonomy is convenient, and it is also contestable, and students should be able to do more than reproduce it. They should be able to state what each domain assumes about the nature of reality, about the nature of knowledge, and about the moral status of the person; they should be able to identify the theories that fall within each domain and the ones that strain against it; and they should be able to identify the assumptions the taxonomy itself makes, several of which are now contested.

The chapter is organised as follows. Sections 1 and 2 establish what a worldview is and distinguish the ontological, epistemological, and ethical questions that a theory answers. Sections 3 to 5 take the humanistic-existential, cognitive, and behavioural domains in turn. Sections 6 and 7 take the systemic and constructivist domains. Section 8 compares the five directly. Section 9 discusses the limits of the taxonomy and the cultural assumptions embedded in it. Sections 10 and 11 address application and the question of pluralism.

# 1. The Idea of a Worldview in Counseling

### 1.1 The three questions

A philosophical worldview is a general orientation toward reality, knowledge, and the good. In counseling it is best understood as the answer a theory gives to three questions.

**The ontological question: what is real, and what is a person?** Is the person a self-directing consciousness, a thinking system, a behaving organism, a relational position, or a meaning-making participant in a field of language? Are mental states causes, or are they descriptions of underlying processes? Is a disorder a thing, a pattern, a dysfunction in a system, or a story that has organised itself around a difficulty?

**The epistemological question: how can we know?** What counts as evidence about a person's mind? Is the appropriate method controlled experiment, careful observation, the systematic study of individual cases, the analysis of language, or the joint construction of meaning between two people? And what is the standing of the practitioner's judgement relative to the client's report?

**The ethical question: what follows for how we treat people?** If the person is a sovereign chooser, then the practitioner is a consultant who has no special claim to authority. If the person is a behaviour shaped by contingencies, then the practitioner is an engineer whose expertise carries an obligation to change the environment rather than to exhort the individual. If the person is a node in a system, then the practitioner must sometimes work against the individual's stated wish in order to relieve the system. If reality is a co-constructed account, then the practitioner is a collaborator whose task is to help the client revise a story that no longer serves them, and the ethical stakes lie in whose story prevails.

These questions are not settled by research, because they are not empirical. They are settled — provisionally, by communities of practitioners, and by the societies in which those communities operate — by argument, by historical contingency, and by the values of the age. That is why the history of counseling theory cannot be told without reference to philosophy, and why a student who wants to know why a technique works will usually arrive at a philosophical question.

### 1.2 Why this matters clinically

The philosophical questions become clinical questions through the mechanism of attention. A practitioner who holds a constructivist view will hear a young person's account of a "bad attitude" as an account whose meaning is negotiable, and will ask what the attitude is protecting. A practitioner who holds a behavioural view will hear the same account and ask what the attitude does. A practitioner who holds a humanistic view will ask what the attitude is for, from the inside of the young person's experience. A practitioner who holds a systemic view will ask whom the attitude is aimed at.

None of these questions displaces the others, but they compete for the limited attention of a counsellor with forty minutes. And the one that is not asked is the one whose answer goes unexamined.

## 2. The Humanistic and Existential Domain

### 2.1 Ontological commitments

The humanistic-existential domain holds that a person is a **self-determining, embodied, socially embedded agent with an irreducible capacity for choice, meaning, and value**. The person is not reducible to a sum of causes, and the person is not merely a product of social forces. There is a centre to the person — the self, the organising, choosing, valuing consciousness — and that centre cannot be taken away, though it can be suppressed, frightened, or denied.

Two further commitments follow. First, the person is **oriented toward growth**: there is an actualising tendency, in Maslow's formulation, a drive toward the realisation of a person's potential, and pathology is best understood as the obstruction of that drive rather than as the absence of a vital ingredient. Second, the person is **embedded in a world of meaning**: life is not a bare biological process to be optimised but an existence to be interpreted, and the interpretations are the person's own.

### 2.2 Epistemological commitments

Knowledge of a person is obtained by **understanding**, which in the phenomenological tradition means grasping the person's frame of reference from the inside, and which in the clinical tradition is operationally realised as accurate empathic attunement. The practitioner does not know the client better than the client knows herself; the practitioner knows the client *differently*, and this different knowing is the contribution.

The epistemology is therefore hermeneutic in orientation: the truth of a subjective account is not a matter of correspondence with an external fact but of its coherence, its vitality, and its capacity to make sense of experience. This has an important consequence for assessment. The humanistic domain treats the client's account as primary data, not as a report requiring verification — although, as Chapter 48 argues, this position requires care in the assessment of risk and in the evaluation of accounts of harm, where the question of factual accuracy is not a metaphysical question but a safety question.

### 2.3 Ethical commitments

The humanistic-existential domain is the most demanding of the five in its ethical demands on the practitioner, because it locates the source of value in the person rather than in the professional. It produces the ethical requirement of **authenticity** — the practitioner must be present in the relationship as a real person rather than as a technique — and it makes the relationship itself, rather than the method, the primary ethical instrument. The counselor who works in this tradition faces an unavoidable exposure: the client's growth is supposed to be toward greater autonomous functioning, and the counseling relationship is supposed to be a model of that functioning, which means that the counselor's own conduct within the relationship is a matter of professional ethics, not merely of style.

### 2.4 Theories within the domain

Person-centered theory (Chapter 10), existential therapy (Chapter 11), Gestalt therapy (Chapter 12), focusing-oriented therapy (Chapter 13), and, with reservations, emotion-focused therapy (Chapter 37) and motivational interviewing (Chapter 44) fall within this domain. The domain's strengths are its account of the practitioner as a genuine participant in the work, its refusal to reduce the person to a symptom, and its account of growth. Its central weakness is its thinness: the domain's concepts are largely negative — what the practitioner should not do, what the person should not be prevented from doing — and the positive specification of the conditions of change in the major treatments within the domain is comparatively recent and comparatively narrow.

## 3. The Cognitive Domain

### 3.1 Ontological commitments

The cognitive domain holds that a person is a **processor of information who interprets experience and acts on the interpretation**. The decisive commitment is that cognition is a mediating process: the organism does not respond directly to the world but to a representation of the world, and it is in the representation that the determinants of feeling and behaviour are to be found.

Two corollaries follow. First, the person is **rational-capable but not necessarily rational**: the capacity for rational appraisal is a possession of the person, and the maladaptive belief is a misapplication of that capacity rather than an absence of it. Second, the person is **in principle changeable by cognitive means**: if the appraisal is the mediating process, then altering the appraisal should alter the feeling and the behaviour, and this is the mechanism on which the cognitive therapies rest.

A further and less comfortable commitment follows from the centrality of appraisal: the person is, in a strong sense, responsible for the interpretations. This is why the cognitive therapies can be experienced by young people as blaming, and why the ethical handling of cognitive work with young people requires attention to power, to developmental capacity, and to the social determinants of appraisal. Chapter 17 addresses the developmental adaptation of cognitive therapy and Chapter 48 the cultural critique of the assumption that appraisal is individual and freely chosen.

## 3.2 Epistemological commitments

The cognitive domain is **empirical and pragmatic**. Claims about belief and outcome are tested by experiment, by the collaborative empirical session in which the practitioner and the client examine the evidence for and against a belief, and by the outcome data. The epistemology of the domain carries a distinctive practical form: the **collaborative empiricism** that characterises cognitive therapy, in which the client and the practitioner are co-investigators of a set of propositions about which both can, in principle, be persuaded.

This is a strong epistemological position with one significant vulnerability: the "empirical" test of a belief is conducted by a person, in a conversation, with a practitioner who is presumed to be rational and who is not always. Chapter 16 addresses the practice of collaborative empiricism and Chapter 18 the related development in rational-emotive behaviour therapy, where the epistemology is explicitly more Socratic and the test of a belief is whether it is "accurate, useful, and helpful" rather than whether it is empirically corroborated.

## 3.3 Ethical commitments

The cognitive domain produces the ethical requirement of **guided autonomy**: the practitioner teaches, demonstrates, and collaborates, but the client chooses. The Socratic commitment of rational-emotive behaviour therapy, the collaborative stance of cognitive therapy, and the directive elements of behavioural rehearsal are all expressions of the same underlying commitment, that the person retains the final judgement. Where this commitment is honoured, the cognitive therapies are among the most empowering in the profession. Where it is not — where the practitioner argues the client into a belief the practitioner believes — the domain's most serious ethical failure occurs, and it is a failure against the young person's developing autonomy that is not visible in the technique.

## 3.4 Theories within the domain

Cognitive therapy (Chapters 16 and 17), rational-emotive behaviour therapy (Chapter 18), schema therapy (Chapter 19), and — as developments internal to the domain rather than as a separate tradition — the third-wave approaches of acceptance and commitment therapy (Chapter 20), dialectical behaviour therapy (Chapter 21), and mindfulness-based cognitive therapy (Chapter 22). The domain's strength is the most substantial evidence base in the profession. Its central weakness is a tendency to treat the content of thought as separable from the social conditions in which it was formed.

# 4. The Behavioural Domain

## 4.1 Ontological commitments

The behavioural domain holds that a person is a **behaving organism whose current actions are a function of its history of conditioning and whose present environment maintains them**. There is no inner agent whose choices require explanation in terms of reasons; there is an organism, with dispositions and a learning history, in an environment with contingencies. Behaviour is real, is public, and is the appropriate object of scientific study.

The domain's decisive methodological commitment is the **avoidance of mentalism**: the methodological behaviourism of the early twentieth century required that explanations be couched in terms of observable stimulus, response, and reinforcement, and that any appeal to the internal states of the organism be treated as a placeholder for an as-yet-undiscovered stimulus. The methodological behaviourism of the 1950s and 1960s has been substantially revised; contemporary behaviour analysis studies private events as data and accepts that behaviour has a cognitive and emotional phenomenology. What survives is the practice of refusing to explain behaviour by appeal to a construct that has not been operationally defined and operationally tested.

## 4.2 Epistemological commitments

The behavioural domain is **empiricist and experimental**. Knowledge is obtained by controlled observation, by experimental manipulation of antecedents and consequences, and by the systematic manipulation of environmental contingencies. The epistemology carries a strong methodological conservatism: the burden of proof lies with the person who asserts that something is internal, and the demonstration of a functional relationship between a context and a behaviour is treated as the strongest available warrant for a clinical claim.

The domain's epistemological conservatism has two faces. It has been the source of the profession's most methodologically rigorous traditions, and it has also been the source of a persistent difficulty with internal experience — a difficulty that Chapter 23 addresses directly, and one on which the contemporary reconciliation attempts discussed in that chapter turn.

## 4.3 Ethical commitments

The behavioural domain produces the ethical requirement of **attention to consequences**. If behaviour is maintained by consequences, then the practitioner has an obligation to examine the consequences in the young person's environment with the same attention one would give to the consequences of an intervention. The behavioural domain is therefore, at its best, the most radical of the five in its implications for the total environment of the child: it locates the site of change in the contingencies rather than in the child, and this is

the ethical as well as the practical significance of the behavioural commitment to environmental design.

This is also where the domain's sharpest ethical difficulties arise. The behavioural framework has historically been used to modify the behaviour of people who have no voice in the decisions that affect them, and the history of applied behaviour analysis includes a substantial record of coercive practice, particularly in institutional settings for people labelled as disabled. The contemporary discipline of the field — the emphasis on the least restrictive procedures, on the assessment of function before intervention, on the young person's participation in goals, and on the professional's obligations to protect the rights of the individual against the institution — is a response to that history, and it is examined in Chapter 24 and in Part VI.

### 4.4 Theories within the domain

Behaviour therapy in its general form (Chapter 23), applied behaviour analysis (Chapter 24), exposure-based and skills-training approaches (Chapter 25), operant conditioning and contingency management (Chapters 26 and 28), behavioural activation (Chapter 27), and the behavioural elements of dialectical behaviour therapy. The domain's strength is its measurability, its explicit mechanism, and the possibility of replication. Its central weakness is the reductive treatment of meaning, of relationship, and of the reasons for which a young person does what they do.

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## Further Reading for This Chapter

- Tatum, C. E. (2017). *Theoretical foundations of contemporary counseling and psychotherapy*. Wiley. — Its opening chapter sets out the philosophical differences among the orientations in the clearest terms available.
- Pope, S. J., & Vasquez, M. J. T. (Eds.). (2021). *Theories of counseling and psychotherapy* (6th ed.). Pearson. — The introductory chapter on "theoretical orientation and worldview" is the standard treatment.
- Raskin, M. (2010). *AmericanDSM: The making of the DSM-III*. — For the advanced student, a history of the diagnostic system and of the battles over the meaning of disorder that the cognitive domain inherited from its mid-century predecessors.
- Held, B. S. (2006). *The varieties of experience in psychological science*. — A philosophical treatment of the several senses of "experience" and their implications for the therapeutic relationship.

## 5. The Systemic Domain

### 5.1 Ontological commitments

The systemic domain holds that a person is a **position within a system of relationships**, and that the system — not the individual — is the primary unit of reality and of intervention. The commitments are circularity, wholeness, and context.

**Circularity** means that causes are reciprocal: a young person's behaviour influences the adults around them, and the adults' responses influence the young person. There is no unidirectional causation of the kind the

behavioural account presupposes; there is a loop. The most famous illustration is the double bind, in which a child receives simultaneously contradictory messages and is placed in the position of responding to either one with distress.

**Wholeness** means that the whole is more than the sum of its parts: the family is not a collection of individuals who happen to live together, but an organisation with its own properties, its own stability, and its own tendency to maintain itself. The concept of homeostasis — the system's tendency to restore its previous state when disturbed — is the foundational idea here, and it explains why families so often resist change even when the change would benefit every member.

**Context** means that behaviour is a function of the system rather than of the person, and that the system is itself embedded in wider systems. The ecological formulations of Bronfenbrenner, discussed in Chapter 50, and the clinical formulations of the family therapies in Part V, are both expressions of this commitment.

## 5.2 Epistemological commitments

The systemic domain is **pragmatic and circular in its method**: the observer is inside the system being observed, and the observation itself is a communication that changes the system. This is the cybernetic insight, and it has a striking clinical corollary. A therapist who observes a family in interaction is not taking a photograph; the observation is a message, and the family will respond to it. A therapist who attempts to remain outside the system is not thereby neutral; the attempt itself is a systemic act.

The methodological consequence is that the systemic domain is unusually comfortable with hypotheses that are provisional and that are tested by what happens when they are acted upon. A circular formulation — "the young person's withdrawal is met by parental chasing, which increases the withdrawal" — is not a linear hypothesis that can be confirmed by a single observation; it is a pattern that is confirmed by the pattern's persistence and by the change that follows intervention at any point in the loop.

## 5.3 Ethical commitments

The systemic domain generates the most difficult ethical questions in the profession, because it is willing to prioritise the system over the individual. If a young person's symptom is serving a family function, then the symptom may be more valuable to the family than the young person's recovery. If a family's equilibrium is maintained by a member's illness, then the request for treatment is, in part, a request for a different equilibrium, and the practitioner may be the agent of that change whether or not the family has asked for it.

Three ethical requirements follow, and the profession's codes and the child welfare literature are largely continuous with them. First, the practitioner must be clear about whose interests are being served and must say so openly. Second, the practitioner must not impose a definition of the problem on a family without the family's participation, which is the ethical point on which the systemic tradition is most sharply criticised and the point at which the narrative and collaborative approaches in Part V were developed. Third, the practitioner must protect the interests of any individual member who is at risk of being sacrificed to the family's or the institution's equilibrium — which in the contemporary child welfare context is a legal as well as an ethical requirement.

## 5.4 Theories within the domain

Family systems theory in the Bowen tradition (Chapter 30), structural family therapy (Chapter 31), strategic family therapy and communication theory (Chapter 32), family therapy with parents and adolescents

(Chapter 33), attachment theory and attachment-based therapy (Chapter 34), ecological systems theory and resilience theory (Chapter 50), and, with modification, the contemporary systemic elements of the solution-focused and narrative approaches (Chapters 35 and 36). The domain's strength is its attention to context, to the patterns rather than the events, and to the multiple systems in which a young person lives. Its central weakness is its tendency to lose the individual in the system, and the tendency of systemic clinicians to be so impressed by the pattern that they fail to notice a member who is being hurt.

# 6. The Constructivist Domain

## 6.1 Ontological commitments

The constructivist domain holds that **reality is partly constructed through language and interpretation, and that the person is a meaning-making participant rather than a transparent locus of causes**. Its ontological claim is not that everything is a dream, and the discipline's practitioners are often at pains to distinguish a serious position from a relativism. The serious position is that there is a difference between the world as it is independently of human construal and the world as it is construed by a particular person, and that much of what matters clinically is the second: the meaning of an experience, the identity it supports, the future it makes possible or forecloses.

The domain's commitment to the *social* construction of meaning is stronger still. What counts as a problem, who is positioned as the disordered one, and what counts as an adequate explanation are all determined within a communicative context, and are therefore open to negotiation. This is the domain's most important theoretical contribution to contemporary counseling, and it is the foundation of the social justice commitments examined in Chapter 48.

## 6.2 Epistemological commitments

The constructivist domain is **hermeneutic and dialogic**. Knowledge is the result of a conversation in which both parties are changed, and the practitioner does not possess the truth about the client. The domain is therefore incompatible with the diagnostic authority of the medical model and with the "collaborative empiricism" of the cognitive domain, since there is no set of propositions about which both parties can be empirically persuaded. What there is instead is a set of accounts that can be examined for their coherence, their usefulness, and their consequences.

The methodological expression of this commitment is the **narrative interview**: the practitioner asks questions whose purpose is not to extract information but to loosen the hold of a dominant story and to make alternative accounts available. Chapter 36 develops this in detail.

## 6.3 Ethical commitments

The constructivist domain generates the ethical requirement of **voice**: the person must be able to tell their own story and to be taken seriously in doing so, and the practitioner must be attentive to whose account is privileged in the setting. The domain's most demanding ethical implication is its treatment of power: if all accounts are constructions, then the practitioner's account — the clinical formulation, the diagnosis, the plan — is a construction too, and it carries power because it is institutional. The requirement is therefore not only that the practitioner be aware of the power but that the practitioner work actively to distribute it.

This is a substantial commitment, and it is not satisfied merely by adopting a constructivist vocabulary. A practitioner can use the language of narrative and co-construction while retaining, in practice, total control of the session's agenda, and the discrepancy between the two is a matter for supervision. Chapter 48 sets out the power analysis that makes the discrepancy visible.

### 6.4 Theories within the domain

Narrative therapy (Chapter 36), solution-focused brief therapy in its stronger readings (Chapter 35), social constructionist approaches to diagnosis and to family therapy, and the postmodern and critical components of the multicultural and feminist theories discussed in Chapter 48. The domain's strength is its capacity to make the invisible visible, particularly the ways in which problems are produced by social arrangements and then attributed to individuals. Its central weakness is its difficulty with the practical and the physical: it is easy, in a constructivist framework, to lose sight of the young person who is not eating, is not sleeping, is cutting, or is not attending school.

## 7. Comparing the Five Domains Directly

**Core Worldview Assumptions Compared Across Domains**

| Domain | Nature of the person | Source of difficulty | Locus of change | Role of the counsellor |
|---|---|---|---|---|
| Humanistic | Organically whole, tending toward growth | Incongruence between self experience and the organism | Within the client | Facilitating conditions |
| Cognitive | A processor of information and appraisal | Maladaptive beliefs and automatic thoughts | Interpretation and belief | Collaborative empiricist |
| Behavioural | An organism shaped by contingencies of reinforcement | Learning history and environmental control | Observable behaviour and context | Engineer of contingencies |
| Systemic | A relational position within a network | Patterns of interaction and communication | The pattern, not the person | Reframing collaborator |
| Constructivist | A meaning-making participant | Language, discourse, and the limits of a single truth | The collaborative account | Co-author of narrative |

*Differences among domains are differences about what is real, not merely about technique.*

**Figure 3.7** Core worldview assumptions compared across the five domains of counseling theory.

| Question | Humanistic | Cognitive | Behavioural | Systemic | Constructivist |
|---|---|---|---|---|---|
| The person is... | a self-determining, valuing agent | an information processor and interpreter | an organism shaped by contingencies | a position within a relational system | a meaning-making participant |
| Difficulties arise from... | obstructed growth and inauthentic conditions | maladaptive beliefs and appraisals | learning history and environmental control | patterns of interaction and circularity | dominant stories and constrained possibilities |

| Question | Humanistic | Cognitive | Behavioural | Systemic | Constructivist |
|---|---|---|---|---|---|
| Difficulties are maintained by... | incongruence and a lack of unconditional regard | the continuing operation of automatic thoughts and core beliefs | contingencies that reinforce the behaviour | self-perpetuating interactional sequences | the power of a narrative and the absence of alternatives |
| The site of change is... | within the client, given favourable relational conditions | the client's interpretation and belief | the environment and the contingencies | the pattern across the system | the client's account and the discourse surrounding it |
| The practitioner's role is... | facilitator of conditions | collaborative expert and teacher | engineer of contingencies and coach | consultant to the system and re-framer | collaborator and co-author |
| The practitioner's authority is... | equal and personal | expert but shared | expert by training | expert in process | equal and negotiated |
| The main ethical danger is... | manipulation by relationship | blaming the client for their appraisals | coercive control of behaviour | sacrificing the individual to the system | appropriation of the client's voice |

Reading across the rows, two observations follow. The first is that the differences among the domains are differences about what is real, not differences about which technique is preferable. The second is that each domain's characteristic ethical danger is the shadow of its characteristic strength. A counsellor who is expert by training in the behavioural domain can control a young person's environment; one who is equal and personal in the humanistic domain can shape a young person without the young person noticing; one who is expert in the systemic domain can override a family's stated wishes; one who is a collaborative teacher in the cognitive domain can argue a young person out of a belief the counsellor believes; one who is a co-author in the constructivist domain can appropriate the client's story. Chapter 1 argued that theory is necessary; this table is the reason it is necessary with care.

## 8. The Five Domains as a Taxonomy, and Its Limits

The five-domain scheme is an instrument, and like all instruments it distorts as well as illuminates. Four distortions deserve naming.

### 8.1 The domains are not mutually exclusive

Cognitive therapy contains behavioural elements; behavioural therapy contains cognitive elements; all the family therapies contain psychodynamic elements; and the third-wave approaches deliberately blur the boundaries. The domains are best treated as centres of gravity rather than as territories, and a theory's location in the taxonomy should be stated with a degree of tentativeness.

### 8.2 The taxonomy is anachronistic

It was constructed in the 1960s as a description of the then-current state of the field, and it has been used since as though it were a natural structure. The most important developments in contemporary counseling — the trauma-informed and somatic traditions, the third-wave cognitive approaches, the attachment-based and relational therapies, the social justice and intersectional commitments — do not fit neatly into any of the five.

### 8.3 The taxonomy is culturally situated

The five domains are all Western, all modern, and all built on an implicit model of the person as an individual. The systemic and constructivist domains push against this most explicitly, and the ecological formulations of Bronfenbrenner in Chapter 50 push further still, but the taxonomy itself takes the individual as the unit of analysis. Chapter 8 develops this critique, and Chapter 50 addresses the alternative.

### 8.4 The taxonomy under-describes the emotional and the somatic

There is no "affective" or "embodied" domain, and the consequence is that the emotion-focused and somatic traditions — developed in the 1980s and 1990s and now among the most influential in the field — have no proper place in the scheme. The omission is a symptom of the taxonomy's origin in a period when affect was regarded as a by-product of cognition and of behaviour rather than as a domain of theory in its own right.

## 9. Applying the Framework: A Single Case Through Five Lenses

Consider a fifteen-year-old boy, Emeka, who has stopped attending school, has been suspended twice for aggression, and whose mother has been called to the school four times in a term. A therapist trained in each domain would frame the case differently.

The **humanistic** therapist asks what Emeka's life is like, what he values, what has been lost, and what he would want different. The dominant question is one of contact: has anyone in this boy's life recently been curious about him?

The **cognitive** therapist asks what Emeka thinks about himself, what he believes about school, and what predictions he makes about what would happen if he returned. The dominant question concerns the accuracy and flexibility of his appraisals and the behavioural predictions that follow from them.

The **behavioural** therapist asks what happens immediately before and after each refusal, what the avoidance does for Emeka, and what the adults around him are inadvertently reinforcing. The dominant question concerns the contingencies.

The **systemic** therapist asks what the school's escalation and the mother's increased attention are doing to the family system, and whether Emeka's absence is holding something together at home that would otherwise come apart. The dominant question concerns the pattern.

The **constructivist** therapist asks whose story is being told — is Emeka the problem, or is the problem the story in which a fifteen-year-old black boy who has been suspended is presumed dangerous? — and what accounts of his behaviour are available that the school has not considered. The dominant question concerns discourse and power.

Each of these is a real question, and the domains produce genuinely different first actions. Notice also that two of them — the systemic and the constructivist — produce an immediate ethical suspicion of the referral itself, while the behavioural produces an immediate assessment task, the cognitive a formulation task, and the humanistic an engagement task. The different first actions are the practical content of the philosophical differences.

## 10. Pluralism with Judgement

The framework described in this chapter has an obvious use and an obvious danger. The obvious use is that it enables a student to locate a new theory quickly: ask what the theory assumes about the person, about the source of difficulty, and about the mechanism of change, and its place in the field becomes clear. The obvious danger is that the framework can be used to license an uncritical pluralism in which any theory may be applied to any problem, which is a form of the theoretical nihilism that Chapter 1 criticised.

The resolution is a pluralism with judgement, and it has three components. First, the practitioner selects theories on the basis of the case: the problem, the person, the setting, the stage of work, and the evidence. Second, the practitioner states the reasons for the selection, and states what the selected theory will not be able to see. Third, the practitioner remains alert to the possibility that the selection is wrong, and builds a review point into the plan at which the question can be revisited on the evidence rather than on the pressure of the moment.

A theory is a lens. A lens is chosen to see something, and the competent use of a lens includes knowing what it makes invisible.

## 11. Key Terms

**Key Terms at a Glance**

- **Worldview** — A general orientation toward reality, knowledge, and the good, expressed in a theory as its ontological, epistemological, and ethical commitments.
- **Ontology** — A theory's account of what exists and what a person is.
- **Epistemology** — A theory's account of how knowledge about a person may be obtained and what counts as evidence.
- **Metaphysics** — The branch of philosophy concerned with the nature of reality; in counseling practice, the assumptions a theory makes about reality that cannot be established empirically.
- **Circularity** — The systemic principle that causes are reciprocal and that sequences of interaction are self-perpetuating.
- **Wholeness** — The systemic principle that a system is more than the sum of its parts and has properties of its own.
- **Homeostasis** — The tendency of a system to maintain its existing state when disturbed, and the mechanism by which families resist change.
- **Double bind** — A communication in which a person receives mutually contradictory messages and is placed in the position of responding to either with distress.
- **Actualising tendency** — Maslow's term for the drive toward the realisation of a person's potential; the core premise of the humanistic account of development.
- **Incongruence** — Rogers' term for the discrepancy between a person's self-concept and the living experience of the person; the condition against which therapy is directed.
- **Methodological behaviourism** — The twentieth-century practice of explaining behaviour only in terms of observable stimulus, response, and reinforcement, and of treating appeals to internal states as placeholders for undiscovered stimuli.
- **Avoidance of mentalism** — The behavioural commitment to requiring operational definition and empirical test before a construct is used explanatorily.
- **Collaborative empiricism** — The cognitive therapy practice of examining the evidence for and against a belief jointly with the client.
- **Guided autonomy** — The cognitive domain's ethical commitment that the practitioner teaches and demonstrates while the client retains the final judgement.
- **Mindless isomorphism** — The constructivist critique of the tendency of one domain's categories to be applied to another without examination; the assumption, for example, that what is true of individual distress is true of a family.

**Key Terms at a Glance (continued)**

- **Narrative interview** — The constructivist interview form whose questions are designed to loosen the hold of a dominant account and to make alternatives available.
- **Centre of gravity** — A description of the five domains as areas of emphasis rather than exclusive territories.
- **Pluralism with judgement** — The professional stance of selecting theories on the case, stating the reasons, and specifying what the selection will obscure.
- **Epistemic relativism** — The position that all accounts are equally valid; the constructivist domain's practitioners generally distinguish their position from this, and the distinction is examined in Chapter 48.

## 12. Chapter Summary

Every counseling theory rests on a philosophical worldview expressed as three commitments: an ontology about what a person is, an epistemology about how that person may be known, and an ethical position about what follows for how the person is to be treated. These commitments are not settled by research, because they are not empirical, and they determine the theory's account of difficulty, its mechanism of change, and its technique.

The five-domain taxonomy — humanistic, cognitive, behavioural, systemic, constructivist — organises the field and has a durable usefulness: it enables a new theory to be located quickly and it exposes the assumptions a technique carries. Its usefulness is bounded. The domains are not exclusive, the taxonomy is anachronistic, it is culturally situated in a Western model of the individual, and it lacks a place for the affective and somatic traditions that have become central in the contemporary field.

The five domains produce different first actions on the same case, and each domain's characteristic strength is also the source of its characteristic ethical danger. The professional task is therefore not to choose a domain and inhabit it, but to choose on the basis of the case, to state the reasons, and to know what the choice makes invisible.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish the ontological, epistemological, and ethical questions a counseling theory answers, and give an example of each in the theory of your choice.
2. Explain the concept of circularity in the systemic domain, and show how it changes the question a behavioural therapist would ask about a young person's school refusal.
3. What does the concept of "guided autonomy" require of a practitioner working with a young person? How might it be violated in practice without the practitioner intending to violate it?
4. Compare the ontological commitments of the humanistic and the constructivist domains. Both attend to meaning; how do they differ?
5. Give an example of a clinical decision in which the constructivist domain would make a practitioner suspicious of the referral itself rather than of the client.
6. What does it mean to say that each domain's ethical danger is the shadow of its strength? Illustrate with two of the domains.
7. Criticise the five-domain taxonomy. What would a better taxonomy look like, and what would be gained and lost by replacing it?
8. Using the Emeka case, write the first three questions you would ask from each of the five positions, and then identify which single question you would ask first and why.
9. What is the difference between "pluralism" and "pluralism with judgement"? Give a clinical example of pluralism without judgement.

## 14. Practice Activities

**Practice and Application**

### Activity 3.1 — The Assumption Ladder (90 minutes, individual then pairs)

Choose one theory you have studied. Write down, in order, the ten assumptions it makes about the world, about people, and about change, from the most general to the most specific. For each assumption, note whether it is (a) empirically testable, (b) a value commitment, or (c) a definition. Then, in pairs, exchange and identify the assumptions your partner's theory shares with yours and those it does not. The value of the exercise is the third category: most students are surprised to discover how many of their most confident theoretical claims are definitional.

### Activity 3.2 — The Same Case in Five Voices (2 hours, groups of five)

Each group member takes one domain and, using the Emeka case, records: the question they would ask first, the hypothesis they would form, the intervention they would propose, what they would be worried about, and the one thing their approach would make it difficult for them to see. Read all five aloud. The group then identifies the two domains that are furthest apart and argues which of the two has the stronger claim on this particular case, with reasons.

### Activity 3.3 — The Assumptions Audit (2 hours, individual, confidential)

List the ten assumptions you personally bring to counseling — about children, about parents, about compliance, about honesty, about the causes of distress, about who is helped and who is harmed, about the value of talking, about the role of discipline, about the nature of expertise, and about what counts as improvement. For each, note where you acquired it and whether you have ever tested it against experience that contradicted it. This is the raw material for the work on cultural humility in Chapter 8 and Chapter 48, and it is better begun in a written form than in conversation.

## 15. Annotated Reading Pathways

**For the philosophical foundations.** Read the opening chapters of the standard survey texts on worldview and theory comparison, and then read one primary philosophical text for the tradition that most repels you — an account of behaviourism for the student who is drawn to the humanistic domain, a phenomenologically informed account for the student who is drawn to the behavioural. The purpose is not to be persuaded but to be able to state the strongest version of a position you do not hold.

**For the domains in their historical form.** The domain literature is best read in the order the domains matured: behaviourism, then psychoanalysis and humanism, then systems, then constructivism. Reading them out of order gives a distorted picture of the field, because each domain was largely a response to the one before it.

**For the critique of the taxonomy.** Read the material on the cultural embeddedness of the individual, on the limitations of the dyadic therapy model, and on the contemporary development of affective and somatic theory. The most productive question to carry is whether a taxonomy organised around the person can adequately represent a field in which the affective, the somatic, and the social are increasingly treated as constitutive rather than as incidental.

**For practice.** Return to the five questions of Activity 3.2 with a real case from placement. Write the first intervention in five registers and then decide, in writing, which you would use and what you would be giving up.

# Practice Laboratory and Extended Worked Example

This laboratory turns **Philosophical Worldviews: The Five Domains of Theory** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is humanistic, cognitive, behavioral, systemic, and constructivist assumptions examined side by side. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Priya is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Priya's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Priya explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Priya chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Priya has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Priya can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Philosophical Worldviews: The Five Domains of Theory

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Philosophical Worldviews: The Five Domains of Theory** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Philosophical Worldviews: The Five Domains of Theory** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

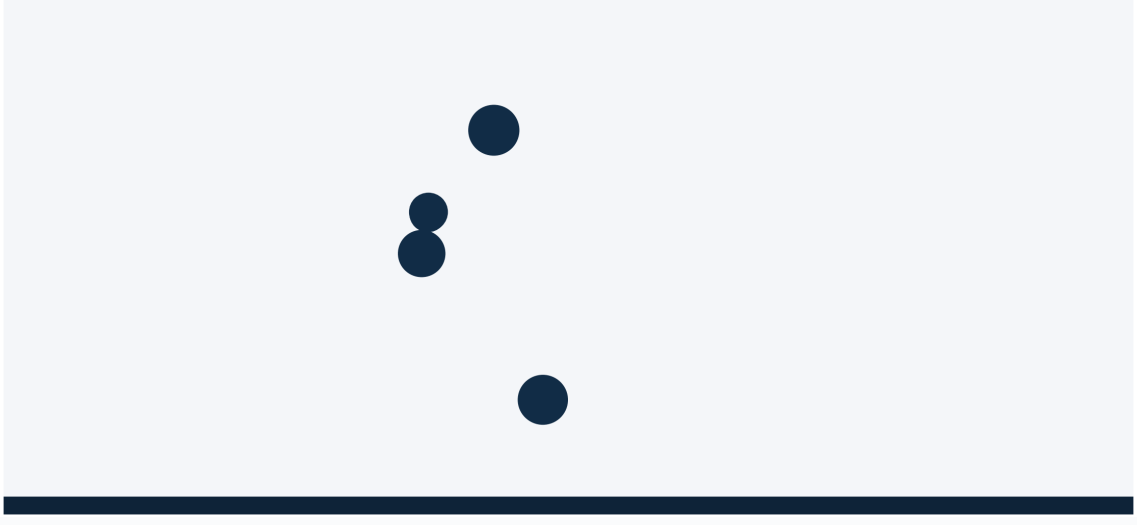
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Philosophical Worldviews: The Five Domains of Theory**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Philosophical Worldviews: The Five Domains of Theory to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 4

# A Genealogy of Counseling Theories

A historical map from the late nineteenth century to the contemporary integrative era.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain what a genealogical account of theory does, and distinguish it from a chronological one.
- Trace the principal lines of descent from the late nineteenth-century movements that produced counseling theory to the contemporary field.
- Identify the specific problem or crisis to which each major theoretical movement was a response.
- Explain the mechanism by which a new theoretical movement displaces or incorporates an older one.
- Describe the significance of the "third wave" and the contemporary developments that followed it.
- Draw four general lessons from the history of the field about the development of theory.
- Locate the origins of the ideas and techniques used in the contemporary practice of counseling with children and adolescents.

## Chapter Overview

Theories in counseling have a genealogy, and knowing it is not a scholarly ornament. It is the fastest way to understand why a theory has the shape it has, why it retains the assumptions it does, why it is more comfortable with some problems than with others, and why a technique that its founder developed for one purpose seems to work for another. It also explains the persistence of arguments that would otherwise look irrational, because a great many of the field's quarrels are the continuation, in a new key, of a dispute that was settled — provisionally, and with costs — eighty years ago.

A chronological account lists theories in the order they appeared. A genealogical account does something different and more useful: it shows that each major theory is a response to a specific crisis in the theory before it, and that the residue of that response remains in the theory, visible and often embarrassing, long after the crisis has been resolved. The cognitive therapies, for example, were invented in part to escape the limitations of behaviourism, and they retain the behaviourist emphasis on measurement, on written records, and on the treatment of the client as a rational agent whose errors can be specified. The humanistic therapies were invented to escape the limitations of psychoanalysis, and they retain a conception of the practitioner as a warm, healthy, autonomous person whose own development is the precondition of the other's. The family therapies were invented partly to escape the individual focus of the intrapsychic schools, and they retain a conception of the symptom as a communication.

This chapter traces the field's descent in six periods and draws four general lessons from it.

## 1. What a Genealogical Account Does

A genealogy, in the Foucauldian sense, asks a question that sounds trivial and is not: *what is the specific problem to which this practice, discourse, or theory is a response?* The point of the question is that ideas do not arise from nothing. They arise because something else became untenable, and the shape of the new idea is determined by the shape of the difficulty it solves.

Applied to counseling theory, this question immediately produces several observations that a purely chronological account misses.

**Theoretical succession is not cumulative.** The history of counseling theory is not a story of steady accumulation, in which each theory adds a piece to a growing edifice. It is a story of repeated displacement, in which a new theory takes over the central questions of the field by re-describing them, and the previous theory survives as a specialism or as a set of techniques. Cognitive therapy did not incorporate behaviour therapy; it colonised the territory and left behaviour therapy in the clinic. The third wave did not replace the second; it split the second wave into a wave that embraced acceptance and a wave that embraced mindfulness, and it created an active and continuing dispute about the value of symptom reduction as an outcome criterion.

**Successions are driven by unmet needs, not by refutation.** A theory rarely displaces another because it has been shown to be false. It displaces it because it answers a question that the incumbent has left unanswered — most often a question about a population the incumbent handles badly, a question about the limits of a method in a particular context, or a question about the acceptability of an intervention to the people receiving it. The emergence of the family therapies in the 1960s was substantially a response to the perceived and actual failure of individual therapy with children whose difficulties lay in their families, and of the psy chodynamic assumption that the individual was the proper unit of treatment. The emergence of the trauma-informed and somatic approaches in the 1990s and 2000s was substantially a response to the inadequacy of talk-based therapies with people whose difficulties were organised at the level of the body and of autonomic arousal.

**Every theory carries its prehistory in its techniques.** The techniques of a mature theory usually encode the assumptions of its predecessors, and they are therefore sometimes inconsistent with the theory's own philosophy. The cognitive therapist's thought record requires written, sequential, propositional thought, and it is a profoundly rationalistic instrument; the practice of it, in the presence of a young person who does not think in propositions, is the site of a persistent and unresolved friction. The behavioural therapist's attention to reinforcement schedules is an extraordinarily refined instrument for a person who is not a machine, and the entire history of applied behaviour analysis has been an argument about whether the person is more or less than a machine.

**The genealogy is a resource for ethical reflection.** Because each theory was a response to a problem, the problem remains available as a standard against which to judge the theory's contemporary use. A technique used in a population or a setting for which it was not developed is easier to identify as such when one knows what problem it was developed to solve.

## 2. Precursors, 1880–1920

The theory of counseling has no single ancestor, but it has a small number of convergent nineteenth-century sources.

**The clinical psychology of light.** Pierre Janet's work on automaticity, on the hierarchical organisation of behaviour, and on the phenomenon of psychogenic deterioration established the first systematic attempt to explain psychological difficulty in behavioural and physiological terms, and introduced the concept of the trauma. Janet's work was rediscovered in the 1980s by the trauma researchers who needed a precursor for the idea of the traumatic memory as an unintegrated bodily event.

**The psychology of individual differences.** The work of Francis Galton, Alfred Binet, and James McKeen Cattell established the measurement of individual differences and the notion of the trait, and supplied the technical apparatus — the test, the norm, the statistical comparison — that counseling still uses in

assessment, career guidance, and programme evaluation. Cattell's factor-analytic programme in particular shaped the trait and factor models that underlie the modern instruments used with young people.

**The first laboratory child.** Lightner Witmer established the first psychological clinic in 1896, in the United States, and coined the term "psychological clinic" for the purpose. The clinic is the institutional ancestor of every subsequent setting in which counseling is delivered, and Witmer's method — the study of an individual child, longitudinally, in relation to the problems brought to the clinic — is essentially the method of a modern case formulation.

**The pragmatist philosophy.** John Dewey, William James, and Charles Cooley supplied a philosophy that treated the person as an organism-in-environment, the self as a product of social interaction, and the examination of experience as the primary method of inquiry. Cooley's concept of the looking-glass self is the direct ancestor of the constructivist and social-cognitive emphasis on the social origin of the self; Dewey's concept of the continuity of experience is the direct ancestor of the developmental approaches in Chapter 9.

**The child study movement.** The child study movement of the late nineteenth and early twentieth centuries produced the systematic observation of children's development, the normative milestone approach, and the "normality" concept. It also produced its characteristic methodological weakness, a reliance on small, unselected samples of children whose representativeness was assumed, and it produced the norm-versus-pathology framework that still organises developmental assessment.

**Psychoanalysis.** Freud's *Interpretation of Dreams* (1900) and the *Three Essays on the Theory of Sexuality* (1905) created the first comprehensive theory of the human mind and of psychological difficulty, and the first theory in which the symptom had a meaning. The clinical technique of free association and the method of interpretation produced the foundational observation that the person resists the interpretation as well as seeking it, which became resistance, defence, and later the whole account of the therapeutic relationship in the relational and humanistic traditions.

## 3. The Clinical Movement, 1920–1940

Between the two wars, a generation of clinicians produced the theories that the mid-century counselling profession would inherit.

**Alfred Adler's individual psychology.** Adler relocated the centre of personality from the intrapsychic to the social, insisting that the person is a striving for superiority and that the person's fundamental problem is a feeling of inferiority against which a compensatory lifestyle is constructed. Adler's work supplied the first coherent account of the child as a striving creature with a subjective goal, and the first therapeutic method centred on the reconstruction of the person's style of life through the analysis of early memories. It supplied the developmental interest in the significance of birth order and of sibling position, which appears, in modified form, in the family therapies of the 1960s and in the modern literature on sibling relationships.

**Otto Rank's developmental will.** Rank emphasised the will, the creative force, and the separation anxiety of the child, and his developmental theory of the first three years influenced attachment research and object relations theory. Rank's notion of the "trauma of birth" was wrong in its specifics and enormously productive in its general form: the idea that the developmental process itself is a source of anxiety that must be negotiated.

**The object relations school.** Melanie Klein, Donald Winnicott, and, in the English context, John Bowlby and John Bowlby's colleague Ronald Fairbairn developed a view of the child in which the central issue is the management of dependency on a caregiver and the internalisation of the caregiver as an object. Winnicott's contributions — the holding environment, the transitional object, the true and false self, the facilitation environment — are among the most cited and most practically useful concepts in the psychotherapy literature, and the concepts of the good enough mother and of the true self are quoted by practitioners who have no use for the theory that produced them.

**The interwar behavioural and learning tradition.** Watson's 1913 manifesto asserted that psychology was a purely objective branch of natural science whose goal was prediction and control, and the Law of Effect (Thorndike, 1905) and the acquisition of conditioned responses (Pavlov) supplied the learning theory on which the later behaviour therapies were built. The 1930s saw the work of Skinner on operant conditioning, Mowrer on the two-factor theory of avoidance, and the clinical application of conditioning to phobias by Mary Cover Jones, whose 1924 treatment of Little Peter is the ancestor of every exposure-based therapy in the modern field.

**The child guidance movement.** The child guidance clinics of the 1920s and 1930s, and the publications of the influential Chicago clinic, established the multidisciplinary team, the case conference, the clinical notebook, and the assessment of the child in the family. The "child guidance" model of the 1930s is the ancestor of the contemporary school-based, family-based, and consultation models of service delivery, and the practice of a written case formulation that begins with a family and a school is a direct inheritance.

**Karen Horney and the neo-Freudians.** Horney's account of the neurotic need for safety and of the three neurotic orientations, and the work of Erich Fromm and Harry Stack Sullivan, moved the field toward an interpersonal and sociocultural account of personality, supplied the concept of the "basic anxiety" and of the "selective neglect" of reality, and produced the clinical observation that became the basis of the cognitive therapy of Albert Ellis: that the neurotic's disturbance lies in a demanding and self-defeating style of thinking rather than in the events themselves.

## 4. The Professional Turn, 1940–1960

The 1940s and 1950s are the decade in which counseling became a profession and in which the central split in the field was made visible.

**The conditions.** The returning veterans, the National Mental Health Act of 1946, the establishment of the National Institute of Mental Health in 1949, the founding of the American Personnel and Guidance Association's predecessor organisations, and the 1950s expansion of the school guidance movement all created the demand, the funding, and the institutional infrastructure for a profession.

**The professional bodies.** The American Association of Counseling and Development (founded in 1917 as the National vocational guidance society, and renamed through several iterations) brought the four traditions — school counseling, mental health, vocational, and rehabilitation — into one organisation. The rival body, the American Psychological Association, and the National Association of School Psychologists and the American Orthopsychiatric Association each claimed a portion of the territory, and the jurisdictional disputes of the present day are the direct descendants of the professional arrangements of this decade.

**Client-centred therapy.** Carl Rogers' *On Becoming a Person* (1961) and his 1950s research on the counselling relationship produced the intervention that the field's most important empirical literature would

later study. The theoretical commitments were: that the person has an innate drive toward growth; that the self-concept is a structure that may be incongruent with experience; that the conditions of growth are the psychological conditions of the therapeutic relationship — congruence, unconditional positive regard, and empathic understanding; and that the practitioner's role is a facilitative, non-directive one. The theory's distinctive claim, and the claim that the subsequent forty years of common-factors research would examine, is that the conditions of the relationship are themselves the therapeutic conditions.

**Maslow and the humanistic theory of motivation.** Maslow's 1943 paper on the hierarchy of needs and his later work on self-actualisation supplied the humanistic domain with its developmental account and its rejection of the drive-reduction model of motivation, which held that behaviour is a function of biological need and the reduction of that need. Maslow's claim that there is a higher motivational level at which growth, creativity, and the pursuit of meaning become the primary drivers of behaviour is a claim with real clinical consequences, and it is the ground of the humanistic domain's scepticism about the behavioural account of motivation.

**Behaviour analysis.** Skinner's 1953 *Science and Human Behaviour* and 1957 *Verbal Behaviour* gave behaviourism its mature statement and its most ambitious application, and the applied analysis of behaviour that followed in the 1950s and 1960s produced the token economy, the shaping procedures, the functional analysis, and the systems of behavioural classroom management that Chapter 28 addresses. The 1950s also saw the rise of behaviour modification in the treatment of severe behaviour problems in institutional settings, with the ethical and methodological controversies that Chapter 24 examines.

**The first randomised psychotherapy trial.** The 1961 work comparing client-centred and psychoanalytic therapy, and the 1959 work on neurotic depression treated with drug and placebo conditions, established the methodological template for the psychotherapy research literature: the comparison of two credible treatments, and the demonstration that a placebo condition produced substantial improvement in the absence of the specific ingredient. Chapter 6 treats this literature in detail.

**The stress reaction.** The work of Selye and of the wartime psychiatrists on combat fatigue produced the concept of the stress response, which was to become, forty years later, the physiological foundation of the somatic and trauma-informed approaches of Part VI.

## 5. The Cognitive Turn, 1950–1975

The cognitive revolution in psychology and the parallel development of the cognitive therapies in clinical practice were driven by two problems with behaviourism that could no longer be ignored.

**The first problem was the treatment of cognition itself.** The behaviourist account of language as behaviour under stimulus control could not accommodate the observation that a thought, once available, changes what a person does. Cognitive psychology solved this by treating information processing as the mechanism of behaviour, and clinical practice solved it by treating the content and structure of thought as the appropriate target of intervention. Albert Ellis' rational-emotive behaviour therapy, published in 1962, made the connection explicitly: it applied the ABC model of rational-emotive emotion to the interpretation of events and insisted that the event is not the cause of the emotion, the client's belief about the event is.

**The second problem was the explanatory poverty of behaviourism about common phenomena.** Behaviourism could explain acquisition and extinction with great precision and had little to say about the phenomenon of insight, the phenomenon of expectancy, the phenomenon of the placebo effect, or the

enormous variance in the response to a given treatment. George Kelly's 1955 *Personal Constructs* supplied the most ambitious of the alternative accounts, proposing that each person constructs a personal theory of the world and that psychological difficulty is a consequence of a non-adaptive or incoherent personal construct system. Aaron Beck's work, beginning in the 1950s with depressed outpatients at the Philadelphia VA hospital and consolidated in 1976, translated Kelly's constructs into a clinically usable method.

**The social learning revolution.** Bandura's 1969 *Principles of Behavior* and 1977 *Social Learning Theory* supplied the mechanism that the cognitive therapists needed: the acquisition and maintenance of behaviour is a matter of modelling, observation, and self-efficacy rather than of direct reinforcement alone. Bandura's demonstration that people learn behaviour by observing models without any reinforcement to the observer was a serious challenge to the behaviourist account, and his self-efficacy theory became a mechanism of change in its own right, later revised in the 1990s into the "self-efficacy and coping" model, which Chapter 45 develops.

**Meichenbaum and the cognitive-behavioural integration.** Donald Meichenbaum's 1975 cognitive-behavioural therapy integrated the behavioural procedures with the cognitive restructuring, and it is from this development that the contemporary cognitive-behavioural therapies emerged, including the treatments for anxiety with children and adolescents that are among the most extensively evaluated interventions in the field.

**The status of the cognitive turn.** By the mid-1970s the cognitive therapies had a coherent theory, a manualised method, a testable model of psychopathology, and a research programme that generated the majority of the field's outcome literature. The "cognitive revolution" in psychology, in the sense of the shift of psychology's centre of gravity from behaviour to internal mental representation, had by then become the field's dominant framework, and the cognitive therapies became its clinical expression.

## Further Reading for This Chapter

- McLeod, S. A. (2003). *Counseling and psychotherapy: Theories, practices, and evidence*. — Not a history, but its opening chapter situates the contemporary theories against their origins with unusual economy.
- Frank, J. L. (2015). *Persuasion and healing* (3rd ed.). Johns Hopkins University Press. — For the historical development of the common factors and the therapeutic relationship.
- Kiesler, D. J. (1982). *The historical roots of psychotherapy research*. — For the advanced student, an account of the methodological developments that made therapy researchable.
- Raskin, M. (2010). *AmericanDSM: The making of the DSM-III*. — For the history of the diagnostic system and the political economy of the profession.
- The primary texts cited throughout this chapter, read in the order of the section in which they appear.

## 6. The Systemic Turn, 1950–1975

The systemic turn is the most consequential intellectual development in the history of counseling, and it is the development most often reduced to a list of techniques.

## 6.1 The Palo Alto group

The intellectual origin of the systemic turn was the work of Gregory Bateson and the research group at the Mental Research Institute in Palo Alto in the 1950s. Bateson's group, studying the communication of families of children diagnosed with schizophrenia, made a series of observations that together constituted a revolution in the understanding of human difficulty.

**The double bind** described the situation in which a person receives mutually contradictory messages, at two levels, and is thereby placed in a position in which any response is punished.

**The "I'm OK — You're OK" communication** described a metacommunication — a communication about a communication — by which one party defines the terms on which the other party's messages are to be interpreted. Bateson's insight was that the pathological communication was not in the content of the messages but in the rules governing their interpretation.

**The "everybody's against me" and "the smart one" positions** — a schizophrenic family described in terms of communication roles rather than of intrapsychic conflict.

**The Palo Alto strategic therapy** that followed from this work — Jackson's behavioural problems as learned communication, Haley's therapeutic double bind, Weakley's and Satir's and Minuchin's and Ackerman's divergent developments from the same group — reframed the symptom as a communication to be deciphered and, in the strategic approaches, as a problem to be solved by directive means.

## 6.2 Bowen's family systems theory

Murray Bowen's contribution was different in kind, and more durable. Bowen's genius was to connect the family system to the individual and to do so in a way that could be traced genealogically. His concepts — the differentiation of self, the levels of emotional process, the intergenerational anxiety continuum, the nuclear family emotional process, the family projection process, the emotional triangle, the sibling position, and the Bowen family systems theory as a general framework for the assessment of a person in the context of their emotional system — place the individual inside a multigenerational emotional field, and place that field inside a differentiation continuum ranging from the emotional fusion of the family of origin to the emotional cutoff of a highly differentiated individual.

Bowen's framework also produced the concept of the **genogram**, a three-generation map of relationships, emotional triangles, and boundaries, which is now one of the most widely used assessment tools in the field and is described in Chapter 30.

## 6.3 The structural turn

The structural family therapies of the 1960s and 1970s — Minuchin in Argentina and at Yale, and, independently, Salvador Minuchin's and Virginia Satir's and Salvador Minuchin's and Paul Ackerman's — differed from the strategic approaches in a crucial respect: they were **not** interested in the symptom's communicative function, but in the structure of the family that produced it. Minuchin's structural model described the family as a system with subsystems, boundaries, hierarchies, and rules of interaction, and the therapist's task as the restructuring of the boundaries and hierarchies. Minuchin's account of the "under-involved" and "enmeshed" family and of the developmentally appropriate handling of the adolescent's demand for autonomy from a family that cannot tolerate it is the direct ancestor of the contemporary work on parents and adolescents in Chapter 33.

### 6.4 The consequences of the systemic turn

The systemic turn changed the unit of analysis, the site of intervention, the conception of the symptom, and the training of practitioners all at once. It is also the point at which the profession's enduring ethical difficulties were created, and these are discussed in Chapter 3 and in Chapter 48: the redefinition of the symptom as functional, the possibility of working against an individual's stated wish, and the difficulty of protecting a vulnerable member of a system that is itself being treated.

## 7. The Integrative Turn, 1970–1995

The 1970s and 1980s saw a significant decline in the sectarianism of the 1960s and the emergence of a working pluralism.

**Theoretical integration as a technical problem.** The works of Larry Beauchamp, of William Glasser, of Lester Floor and the eclectic movement, and the work of the integrative theorists who followed them — the technical eclecticism of Arnold Lazarus, the theoretical integration of Wayne Watson and the authors of the 1991 volume on integrating therapies, and the later work of the "psychotherapy integration" research group — attempted to specify the conditions under which two theories could be combined without incoherence. The criteria that emerged, and that Chapter 51 adopts, are: that the integrated approach be grounded in a shared metatheory; that it be based on a shared understanding of the person and of pathology; that it use a shared set of concepts; and that it be based on a shared model of change, including a shared theory of the therapeutic relationship and a shared set of strategies for change.

**The common factors movement.** The clinical and theoretical argument that the common factors were the true determinants of outcome, advanced most vigorously by Jerome Frank and later by Bruce Liese and Bruce Wampold, became in the 1980s and 1990s a major research programme. The finding that the quality of the therapeutic alliance is the most consistent predictor of outcome across therapies and that specific techniques contribute less than their proponents claim was a genuinely important advance, and it was also, as Chapter 6 argues, frequently over-read.

**The training model and the counselselling model.** The 1980s and 1990s saw the consolidation of the two doctoral training models, the counselling psychology model and the clinical psychology model, within a single accreditation framework (the APA's 1995 accreditation standards) and a period in which the professional identities of counselors and psychologists in training became the subject of intense and occasionally bitter dispute. The Council for Accreditation of Counseling and Related Educational Programs, formed in 1994 and first accredited in 1996, created a separate accreditation pathway for the counseling professions, and with it a distinct professional identity with a distinct code of ethics. The history of that development is the history of the "licensure compact" question, and it continues to shape the profession's politics.

**The managed care era.** The arrival of managed care in the 1990s transformed the conditions of practice in the United States and had a substantial effect on theory, because it made short-term, symptom-focused, measurable interventions economically necessary, and because it made the "evidence-based" label a condition of payment. The consequences for theory were direct: the emphasis on specific, manualised, time-limited, outcome-monitored treatment that characterised the 1990s and 2000s was not solely a scientific development. It was also, and in part, a response to the economic conditions of the market.

**The evidence-based movement.** The parallel development of evidence-based practice, the empirically supported treatment movement, and the Cochrane-style systematic reviews of psychotherapy gave the field a genuine empirical foundation and simultaneously contributed to a form of intellectual narrowness that the contemporary field is only beginning to correct. The overstatement of the evidence base has been documented extensively, and the corrections are set out in Chapter 7.

## 8. The Third Wave, 1990–2015

The "third wave" describes a family of developments that began in the 1980s and 1990s and that share a common critique of the second wave: that by treating cognition, behaviour, and emotion as targets to be directly modified, the second-wave therapies had taken the symptoms of psychological difficulty too seriously and had inadvertently reinforced a model in which the self was a set of faulty components requiring correction.

The third wave is best understood as a **reformulation of the relationship between acceptance and change**.

**Acceptance and commitment therapy.** Steven Hayes and colleagues developed ACT from a relational frame theory in which the central human problem is described as "cognitive fusion" — the entanglement of one's thoughts and feelings with the sense of oneself, such that the content of a thought is taken to be the truth of the person, and the resulting struggle with unwanted internal experiences is itself the mechanism that maintains distress. ACT's central metaphor is the struggle switch: efforts to control unwanted thoughts and feelings produce rebound effects that increase rather than decrease distress. The ACT approach is to develop psychological flexibility, defined through six processes: acceptance, defusion, contact with the present moment, the self-as-context, values, and committed action. The clinical relevance of ACT for this course is that it is explicitly hostile to the assumption that symptoms must be reduced, and it has a growing evidence base with adolescents, including in school settings and in the treatment of disordered eating and of substance use.

**Dialectical behaviour therapy.** Marsha Linehan's 1993 manual described a comprehensive treatment for chronic self-harm and suicidality built on two core strategies — dialectical strategies that maximise acceptance and change simultaneously, and validation strategies that make clear to the client that their emotions and responses make sense, while also providing problem-solving strategies that teach new skills. The treatment assumes a client population for whom the usual therapeutic relationship is inadequate, and the behavioural analysis of the suicidal behaviour, the "problem-solving" orientation, and the consultation team model were all innovations with respect to what was standard in the 1990s. Chapter 21 examines the adolescent adaptations.

**Mindfulness-based cognitive therapy and mindfulness-based stress reduction.** Jon Kabat-Zinn's work from the late 1970s at the Massachusetts General Hospital medical clinic, and Zindel Segal, Mark Williams, and John Teasdale's 1990s randomised trial of mindfulness-based cognitive therapy in the prevention of relapse in recurrent depression, brought a distinctive practice into the field — a practice of non-judgemental present-moment awareness — and a distinctive theoretical claim, that changing one's relationship to thoughts is a route to change that does not require changing the content of thought. Chapter 22 addresses both the treatment and its developmental cautions.

**The functional contextual approach.** The functional contextual approach, developed principally by Hayes, Strosahl, and Wilson and given its most systematic clinical expression in the contextual behavioural

therapies, provides the theoretical account of the third wave's central claim: that psychological suffering is a function of the individual's struggle with internal experiences in the context of a life that is not behaving in accordance with the individual's values. The approach also supplies the "five senses of the therapist's behaviour" — evocation, freedom, presence, attunement, and potency — which is a valuable framework for the personal and relational practice of any orientation.

**The third wave's contribution.** The third wave's central contribution to the field has been to establish that the aim of therapy is not the eradication of symptom but the construction of a life worth living, and that a young person may be helped considerably by learning to live with distress that is not to be removed. Its central controversy is whether the emphasis on acceptance undermines the behavioural principle that symptom reduction is itself therapeutic, and the controversy has not been resolved.

## 9. The Contemporary Turn, 2010 to the Present

The most recent period has produced four developments that a graduate student in this course must know.

### 9.1 Trauma-informed and phase-oriented practice

The recognition of the prevalence of traumatic experience, the accumulation of research on the neurobiology of trauma, and the failure of the field to respond adequately to the needs of trauma-exposed populations have produced a distinct body of practice organised around three principles — safety, trustworthiness and transparency, and peer support — and around a phased model of treatment in which stabilisation precedes trauma processing, which precedes integration. The trauma work of Judith Herman, of Bessel van der Kolk, of Pat Ogden, and of the developers of EMDR, of prolonged exposure, of cognitive processing therapy, and of skills-based stabilisation protocols has produced the field's most important methodological advances of the last forty years. Chapters 38, 39, and 40 develop this material.

### 9.2 The somatic and the autonomic

The recognition that many of the presentations that bring young people to counseling are organised at the level of the body — hyperarousal, dissociation, somatic complaints, difficulty with regulation — has produced the somatic, sensorimotor, and polyvagal-informed approaches, and the movement to integrate affect and interoception into the theory of change. The developmental research on the maturation of the prefrontal cortex and of the autonomic nervous system, and the clinical work on the window of tolerance, have provided a neurobiological account that is directly relevant to the sequencing of interventions with young people. Chapters 37 and 43 develop this material.

### 9.3 Social justice, cultural responsiveness, and intersectionality

The reframing of multicultural counseling from a concern with the cultural background of the client to a concern with power, privilege, and the social production of distress has transformed the ethical requirements of the profession. The intersectional framework, the analysis of structural racism in the aetiology of adolescent mental health, the attention to the mental health effects of poverty, of racism, of immigration, of gender and sexual minority stress, and of disability, and the development of a distinctly anti-oppressive practice have made the accountability of the practitioner to the social context a central ethical demand. Chapters 48 and 49 develop this material.

### 9.4 The integration of the digital

Teletherapy, which was a necessity during the global pandemic and which is now a permanent component of the delivery system, has required a generation of practitioners to think about the medium as a theoretical variable: about the loss of the somatic and spatial cues of the in-person encounter, about the security of the home environment, about the presence of parents in the room, about the implications for confidentiality, about digital inequity, and about the question of whether the alliance can be formed in a medium that was never designed for it. Chapter 55 addresses this.

## 10. Four Lessons from the Genealogy

**Lesson one: every theory is a response to a specific problem, and the problem is usually still with us.** The cognitive therapies respond to the limits of behaviourism; the humanistic therapies to the limits of psychoanalysis; the systemic therapies to the limits of the individual focus; the third wave to the limits of symptom reduction as a criterion; the contemporary somatic and trauma work to the limits of the talk-based assumption. A student who can name the problem can predict the theory's blind spots.

**Lesson two: theoretical succession is driven by populations and contexts, not by refutation.** The family therapies emerged because individual therapy did not work well with the problems children actually brought; the third wave emerged because the second wave's methods were experienced by many clients as coercive; the somatic approaches emerged because talk-based therapy did not reach the bodily dimension of many presentations. A theory that is the best available approach for a population and a context is likely to be superseded by a theory that suits a different population or a different context, and the superseded theory is not thereby wrong.

**Lesson three: the techniques outlive their philosophies, and this is both a strength and a danger.** The most useful techniques in the field — exposure, behavioural rehearsal, thought records, the empty chair, the genogram, the miracle question, grounding — are used by practitioners of many orientations, and the portability of these techniques is a practical blessing. It is a danger because a technique detached from the theory that made it effective often lacks the rationale that made it work, and because a technique used outside its developmental range can be ineffective or harmful.

**Lesson four: the profession's recurring error is a claim of aetiological completeness.** Each new theory has tended to present itself as the correct account of the whole of psychological difficulty, and each has been shown to be an account of some part of it. The contemporary field's most mature position is the pluralistic and integrative one developed in Part VIII, and this chapter's history is the reason for it: the evidence of the genealogy is that every single-system account has been partial.

## 11. Key Terms

**Key Terms at a Glance**

- **Genealogy** — An account of the descent of a theory or practice that asks what specific problem it is a response to, rather than simply when it appeared.
- **Residue of succession** — The visible retention in a later theory of assumptions inherited from the theory it displaced.
- **Theoretical succession** — The displacement of a dominant theory by a new one, generally driven by an unmet need rather than by refutation.
- **Prehistory of technique** — The philosophical assumptions encoded in a therapeutic procedure, which remain operative after the procedure is transferred between theories.
- **Moral treatment** — See Chapter 2; the nineteenth-century doctrine underlying the profession's founding moral claim.
- **The looking-glass self** — Cooley's concept of the self as constituted through the imagined perceptions of others; an ancestor of the constructivist account of identity.
- **Law of effect** — Thorndike's principle that behaviours followed by satisfying consequences are strengthened; the foundation of the operant tradition.
- **Two-factor theory of avoidance** — Mowrer's account of avoidance behaviour as acquired through classical conditioning and maintained by negative reinforcement; the foundation of the modern exposure therapies.
- **ABC model** — Ellis's rational-emotive model in which the activating event leads through beliefs to consequences, with the event itself not being the cause of the emotional consequence.
- **Personal constructs** — Kelly's theory that each person constructs an idiosyncratic personal theory of the world through a process of bipolar alternatives, and that psychological disorder is a consequence of a non-adaptive construct system.
- **Double bind** — Bateson's account of a communication in which a person receives mutually contradictory messages and is penalised for any response.
- **Metacommunication** — A communication about a communication, which in the Palo Alto formulation carries the operative rules of a relationship.
- **Family projection process** — Bowen's account of the transmission of emotional processes from parents to children.
- **Differentiation of self** — Bowen's concept of the degree to which a person maintains a distinct sense of self while remaining emotionally connected to others.
- **Genogram** — A multigenerational map of family relationships, emotional triangles, and boundaries, used for assessment.

**Key Terms at a Glance (continued)**

- **Structural family therapy** — The approach, principally associated with Minuchin, that treats the family as an organisation with subsystems, boundaries, hierarchies, and rules.
- **Third wave** — The family of therapies from the 1990s onward that reformulated the relationship between acceptance and change, including ACT, DBT, and mindfulness-based approaches.
- **Cognitive fusion** — The ACT concept of the entanglement of thought with the sense of self that makes the content of a thought feel like the truth of the person.
- **Psychological flexibility** — The ACT construct describing the capacity to remain in contact with present experience and to act in accordance with values while unwanted internal experiences are present.
- **The five senses of the therapist's behaviour** — Evocation, freedom, presence, attunement, and potency; a framework from the functional contextual approach for the personal conduct of the practitioner.
- **Window of tolerance** — The range of arousal within which a person can engage in flexible, integrated response, above which the person is hyperaroused and below which the person is hypoaroused.
- **Polyvagal theory** — The contemporary neurophysiological account of the three-tiered autonomic response to threat, and of the social engagement system.

## 12. Chapter Summary

A genealogical account of counseling theory asks not when a theory appeared but what problem it answered. Read this way, the history of the field is a sequence of displacements, each driven by an unmet need rather than by refutation, and each leaving a visible residue in its successor.

The field descends from the late nineteenth-century movements in clinical psychology, the measurement of individual differences, the child study movement, pragmatist philosophy, and psychoanalysis; it acquired its professional form between 1940 and 1960; it divided along the humanistic–behavioural axis in the 1950s; it was transformed by the cognitive turn, the systemic turn, and the integrative turn; and since 1990 it has been reshaped by the third wave, by trauma-informed and somatic practice, by social justice and intersectionality, and by the digital delivery of care.

Four general lessons follow: every theory answers a specific problem that is usually still with us; succession is driven by populations and contexts; techniques outlive their philosophies, which is both a practical blessing and a danger; and the profession's recurring error is the claim that any single account is complete. That last lesson is the historical justification for the integrative stance developed in Part VIII.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. What is the difference between a chronological and a genealogical account of theory, and what does the second reveal that the first conceals?
2. Identify the specific problem to which the following were responses: the family therapies, the third wave, the trauma-informed approaches, and the social justice critique. For each, say what the response retains from the position it displaced.
3. Explain the double bind and the concept of metacommunication, and state how each differs from an intrapsychic account of the same family interaction.
4. How did the managed care era shape counseling theory, and why is it important to distinguish that influence from the influence of the research literature?
5. In what respects is the third wave's claim about acceptance a repudiation of the behavioural principle, and in what respects is it an extension of it?
6. What is the "residue of succession" in the contemporary practice of cognitive therapy? Use a concrete example from the technique of thought recording.
7. Bowen's system and Minuchin's structure both reformulated the family. State the central difference between them, and what clinical difference it makes.
8. What did the genealogy contribute to your understanding of why the integrative position in Chapter 51 is the field's mature position rather than a compromise?

## 14. Practice Activities

**Practice and Application**

### Activity 4.1 — The Line of Descent (2 hours, individual, then seminar)

Choose one contemporary technique in your work — thought recording, exposure, the genogram, the miracle question, grounding, or any other. Trace it backwards through at least four steps to the moment of its invention, naming the problem it was invented to solve at each step. Write the account in 800 words. Then state which of those problems is still with you in practice, and what that implies about the technique's likely usefulness to you.

### Activity 4.2 — The Crisis Map (90 minutes, small groups)

Each group takes one of the following crises in the field's history: (a) the failure of psychoanalysis to produce predictably effective treatment; (b) the behavioural revolution and the resistance to it; (c) the child guidance movement's encounter with the family; (d) the "common factors" finding and the nihilistic reading of it; (e) the managed care era; (f) the trauma literature's challenge to the talk-based therapies. The group produces a one-page map with: the crisis, the response, the residual assumptions, and the blind spot of the response. Present in ten minutes.

### Activity 4.3 — A Teleological Reading of Your Own Field (60 minutes, individual, written)

Write two hundred words on the following: if you were writing a genealogy of counseling theory from 2040, looking back at 2020s practice, what crisis would you identify as the occasion of the theory or practice that will dominate the 2030s? What problem is it answering? What will it retain from the positions it displaces? This exercise is deliberately speculative and should be argued, not predicted; the point is to practise thinking in the mode the chapter has described.

## 15. Annotated Reading Pathways

**For the history itself.** Read the founding texts in chronological order, with attention to what problem each was written to solve. The order that will be most instructive is: Watson (1913), Freud (1900/1905), Adler (1927), Rogers (1942, 1957), Maslow (1943), Skinner (1953), Kelly (1955), Bateson (1956), Ellis (1962), Beck (1976), Minuchin (1974), Linehan (1993), Hayes (2004). Reading them in this order, a student will watch the field's central question — what is a person, and therefore what is a difficulty — being asked and answered in roughly nine different ways.

**For the historiography.** Read the professional history literature rather than the theoretical surveys: the accounts of the founding of the professional bodies, of the accreditation contests of the 1980s and 1990s, and of the managed care era. The most instructive of these accounts combine institutional history with an awareness of the political economy of mental health services.

**For the contemporary turn.** Read the primary works in each of the four contemporary developments, and then read one critical review of each, written by someone in a different theoretical camp. The critical reviews are what turn a reading list into professional judgement.

**For the exercises.** Activity 4.1 is the most valuable of the three. Do it for a technique you actually use, and bring the line of descent to the seminar; the discussion is invariably better than the individual writing.

## Practice Laboratory and Extended Worked Example

This laboratory turns **A Genealogy of Counseling Theories** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is a historical map from the late nineteenth century to the contemporary integrative era. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tunde is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tunde's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tunde explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Tunde chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tunde has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tunde can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for A Genealogy of Counseling Theories

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **A Genealogy of Counseling Theories** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **A Genealogy of Counseling Theories** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

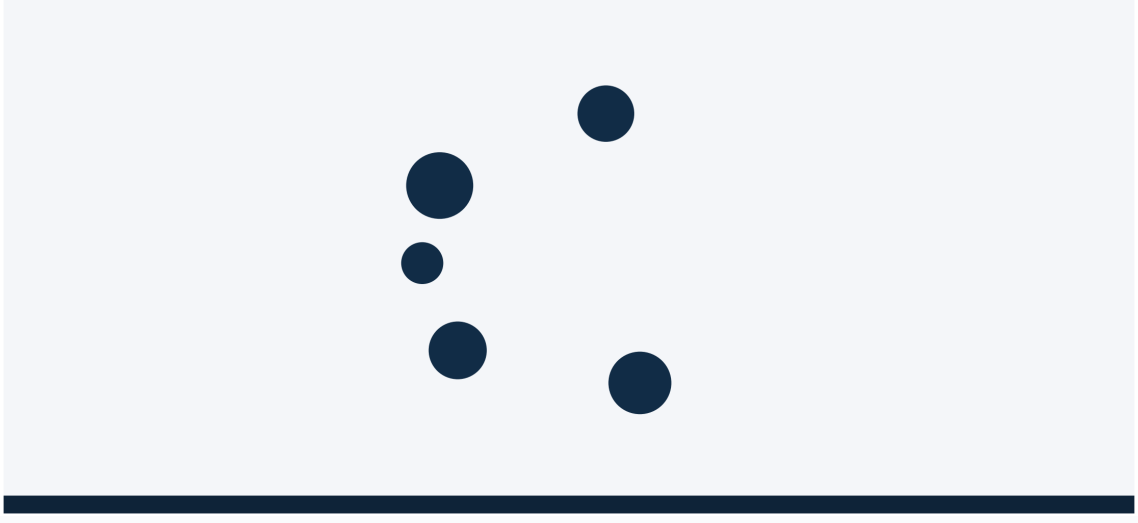
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **A Genealogy of Counseling Theories**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain A Genealogy of Counseling Theories to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 5

# The Therapeutic Relationship: Alliance, Rupture, and Repair

The common substrate on which all theory-specific methods rest.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain why the therapeutic relationship is regarded as a necessary condition of change and how this is compatible with a theory-specific account of technique.
- Distinguish the working alliance, the therapeutic alliance, the rapport, and the therapeutic bond, and describe the function each serves.
- Summarise the empirical findings on the relationship–outcome relationship, including its magnitude, its specificity, and its limits.
- Describe the four principal approaches to rupture and repair, and state the clinical implication of each.
- Apply the rupture-and-repair model to a case involving a young person's withdrawal, silence, or hostility.
- Describe what is known about the alliance with children and adolescents, and why it differs from the adult literature.
- Explain the ethical significance of the relationship, including the specific risks that attend work with minors.

## Chapter Overview

There is a common misreading of the finding that the therapeutic relationship predicts outcome. It is sometimes taken to mean that relationship is everything and technique is nothing, and that the theories surveyed in this book are therefore dispensable. Chapter 6 will take that misreading apart in detail. This chapter takes the finding at face value and draws the correct conclusion from it: the relationship is the medium through which every theory's technique operates, and a theory that does not specify the conditions of a working relationship has left a crucial part of its account of change unspecified.

The argument of the chapter has four parts. First, the therapeutic relationship is a necessary condition of change, and the reasons for this are not mysterious: a young person who does not trust the person they are speaking to will not say the things that need to be said, and no technique survives that failure. Second, the relationship is measurable, and a substantial research literature describes what a good alliance looks like, what predicts it, and what happens when it breaks down. Third, rupture and repair are ordinary and expectable features of the work rather than catastrophic failures, and there is now a well-developed clinical literature on how they are managed — one that differs sharply across theoretical orientations, and whose differences are clinically consequential. Fourth, the relationship with a young person is not the adult relationship, and the developmental adaptations that this requires are specific and teachable.

The chapter includes a substantial case illustration, because the alliance is the one element of counseling practice that every student, whatever their theoretical orientation, will have to get right.

## 1. The Relationship as the Medium of Therapy

### 1.1 The logical argument

Every therapy requires at least two things: a person who wants something to be different, and a set of activities directed at that change. The relationship between the practitioner and the client is the condition under which the second can operate on the first.

The argument is straightforward. A cognitive therapist's method requires the client to record thoughts, to examine them, to generate alternatives, to test predictions, and to report honestly on what happened when they tested them. A behavioural therapist's method requires the client to attempt feared situations and to report accurately on their anxiety. A family therapist's method requires the family to tolerate discomfort and to have the capacity to change an interaction in the room. In each case, the technique depends on information and on cooperation, and the availability of both is a function of the relationship.

This is why the relationship is best understood as a **necessary condition** rather than as a **sufficient cause**. A strong relationship does not by itself produce change; a young person can feel entirely safe with a practitioner and still not change. But the relationship is the condition under which the specific activities of therapy can take place, and this is a proposition about mechanism, not a claim about the irrelevance of theory.

## 1.2 What the relationship contains

The components of the therapeutic relationship are more specific than the phrase suggests, and specifying them is one of the principal purposes of this chapter. Four components are usually distinguished.

**The working alliance** is the collaborative, task-focused dimension of the relationship: the shared agreement about what the work is and what each party will do. It is the most directly relevant to young people, because a young person who does not know why they are in a room and what is supposed to happen will disengage regardless of how much they like the practitioner.

**The therapeutic alliance** is the broader relational bond: the shared trust, the shared belief that the work will be worth doing, and the mutual commitment to it. In the empirical literature, the therapeutic alliance is usually measured by a rating instrument — the Working Alliance Inventory and its revisions, the Session Rating Scale, the Client Outcome Questionnaire, the Therapy Alliance Scale, and the Helping Alliance Questionnaire — and it is this broader construct that correlates with outcome.

**Rapport** is the interpersonal comfort, mutual liking, and ease of interaction. It is the most superficial of the four, and the most important not to confuse with the others. Rapport is pleasant; the alliance is a working partnership; the bond is the affective commitment to the work. A practitioner can have excellent rapport and a poor working alliance, and this is a common configuration with adolescents, who may like a practitioner very much while agreeing to do nothing.

**The therapeutic bond** is the affective and emotional component: the sense of a genuine, caring, reliable connection. In work with young people this is the component most likely to be decisive, because a young person's willingness to disclose depends more on the perceived trustworthiness of the adult than on the perceived competence of the technique.

## 1.3 The relationship in theoretical terms

Each theory in this book specifies the relationship differently, and the specification is part of the theory rather than an addition to it.

The humanistic tradition makes the relationship the treatment. Rogers' hypothesis was that the conditions of growth are interpersonal conditions, and that the "actualising tendency" operates in the relationship or not at all. For a student of this tradition, the relationship is not a vehicle; it is the mechanism.

The cognitive tradition treats the relationship as a necessary condition for the collaborative examination of evidence, and it has been productive enough to generate a research literature on the alliance in cognitive therapy specifically, including the finding that the alliance mediates outcome in cognitive therapy as it does in other treatments.

The behavioural tradition treats the relationship as a behaviour itself, subject to the same contingencies as any other. Behaviour analysts have written a substantial literature on the therapist's behaviour as a reinforcer, on the "rapport" behaviours that increase the young person's verbalisation, and on the fact that the relationship is what makes the contingencies of therapy effective. The tradition's most bracing statement of this position is the claim that the therapeutic relationship is a behaviour, and should be treated with the same rigour as any other.

The systemic and relational traditions make the relationship a **parallel process**: whatever is happening in the relationship between the practitioner and the young person is a reflection of the relational patterns in the young person's wider life. A systemic practitioner working with an adolescent who is compliant with the practitioner and in constant conflict with the mother will read the compliance as data about the family system. The parallel process hypothesis is a powerful lens, and it is also easy to abuse, because it can be used to attribute to the young person a pathology of relationship that is in fact a reflection of the practitioner's own conduct.

The constructivist and narrative traditions make the relationship a **negotiated collaboration** in which both parties are understood to be shaping the account. The practitioner is not a blank screen and the client is not a recipient; the account of what is happening is a joint product, and the question of whose account prevails is itself a matter of power and of ethics.

[Figure placeholder — The therapeutic alliance and the rupture-repair cycle in the development of a working relationship with a young person.]

# 2. The Empirical Literature on the Alliance and Outcome

## 2.1 Magnitude

The relationship between the quality of the therapeutic alliance and outcome is one of the most reliably replicated findings in psychotherapy research. Meta-analyses across adult populations report correlations in the range of 0.25 to 0.30, which is a moderate effect and is large by the standards of psychotherapy research. The finding holds across theoretical orientations, across treatment length, across diagnostic categories, and across practice settings. It holds in the adolescent literature as well, though the adolescent studies are fewer and more heterogeneous, and the magnitudes reported are generally somewhat lower.

## 2.2 Specificity

The finding is also not entirely nonspecific. The alliance-outcome relationship is stronger when the outcome is measured in the same domain as the therapeutic focus; a working alliance focused on the reduction of a specific symptom predicts that symptom's reduction more strongly than it predicts global functioning. And the alliance quality in the early sessions predicts later outcome better than the alliance quality in the middle

or late sessions, which is the basis for the clinical recommendation to invest heavily in the first two or three sessions.

### 2.3 Directionality and cause

A finding of this robustness requires careful handling of the direction of causation. Three explanations are possible: that a good alliance causes better outcomes; that better outcomes cause a good alliance; or that a third variable produces both. The third explanation has the strongest empirical support. The factors that predict a good alliance are largely the same factors that predict a good outcome: the severity of the client's difficulties, the client's prior positive expectations, the therapist's empathic responding and experience, the match between the client and the therapist, the presence of a clear plan, and the client's overall social support. This does not make the alliance causally inert; it makes it a marker of a set of conditions including the therapist's skill, which does cause better outcomes.

### 2.4 The limits

Two limits should be stated. The alliance predicts variance in outcome, and the amount of variance is moderate; it does not predict most of the variance, and a practitioner cannot rely on the relationship alone. And the alliance is not a substitute for technique: in treatments for specific conditions, the specific procedures account for outcomes over and above the alliance, and the evidence from component studies indicates that removing a component while holding the relationship constant reduces outcomes for that component's target.

## 3. The Developmental Specificity of the Alliance with Young People

The adult alliance literature cannot simply be applied to a fourteen-year-old, and the reasons are worth making explicit.

**The developmental constraint on verbal processing.** A young person at the developmental stage at which she is will have a limited capacity for abstract, verbally mediated emotional processing, a limited capacity for metacognitive reflection, and a limited repertoire for articulating what she feels. The alliance researcher's instruments were designed for adults and assume a level of introspective access that a twelve-year-old does not have. This is a measurement problem and it is also a clinical one: the young person's inability to articulate her experience does not indicate a poor alliance.

**The authority asymmetry.** The adult client and the adult therapist are, in most respects, socially equal. The young person and the adult practitioner are not. The alliance with a young person is formed under conditions of considerable institutional power imbalance, which affects what can be said, what must be said, and what is safe to say. The developmental literature on the adolescent's increasing autonomy and the parent's corresponding withdrawal of authority is directly relevant.

**The time horizon.** The adult client is usually able to sustain a therapeutic alliance through a period of discomfort because the abstract possibility of benefit is available to her. A young person's temporal perspective is often shorter and more concrete, and a young person who cannot see a point to a process that is currently painful will disengage. The clinical implication is the importance of making the goal concrete, near, and in the young person's own terms.

**The developmental instability of the "self" the alliance is with.** The young person the practitioner met in the first session is not the same person who attends the eighth; adolescent identity is under active construction, and the alliance may have to be renegotiated in the light of developments outside the sessions. Chapters 46 and 9 address this.

**The third-party presence.** A young person's counselling usually involves adults who are not the client: parents, teachers, school staff, guardians, sometimes other professionals. These figures are present in the alliance as a matter of course, and their presence affects what can be said and what can be trusted. The systemic traditions theorise this explicitly; the individual traditions usually treat it as an external complication.

**The corroboration of the relationship by the environment.** For a young person, the most important evidence that the practitioner can be trusted is that the practitioner's behaviour is consistent with what the young person has been told by the adult in their life. If a practitioner tells a young person that what they say will be kept confidential and then a parent is telephoned without explanation, the alliance is not merely damaged — the young person learns something about the reliability of adults that generalises.

# 4. Rupture: When the Relationship Breaks Down

## 4.1 Defining rupture and repair

A **rupture** is a breakdown in the working alliance: a moment in which the collaborative frame of the therapy is interrupted. It may be overt — the young person refuses to speak, attacks the practitioner, or demands to leave — or it may be covert, expressed through lateness, minimal responses, or the withdrawal of a previously engaged participant. A **repair** is the process by which the collaborative frame is restored, whether by the practitioner alone, by the young person, or jointly.

The clinical significance of rupture is that it is a **reliable indicator of the state of the work**, and its content is **the most direct available information about the young person's experience of the therapy**. A young person who withdraws is telling the practitioner something about the relationship, and the more rapidly the practitioner can identify the rupture and the more accurately the practitioner can interpret it, the less damage is done and the more useful the information becomes.

## 4.2 Four models of rupture

**The psychodynamic model.** Rupture is a manifestation of the transference — the displacement onto the practitioner of the expectations, feelings, and unresolved relationships from the past. The rupture is the enactment of an old relationship pattern in the present, and its interpretation is the heart of the intervention. A young person who becomes angry with the practitioner in the seventh session is asked what it is about the practitioner that makes the anger possible, and the answer, if the work goes well, reaches the earlier relationships in which the young person's anger and the adult's response to it were formed. The psychodynamic model's risk is that it can be used to explain away everything, and a practitioner who interprets every behaviour as transference will eventually find that the young person has stopped being heard.

**The behavioural model.** Rupture is a behaviour to be understood functionally: what does the young person get from it, and what has it been reinforced for? The behavioural practitioner's first move is not to interpret but to change the contingencies: what happened immediately before the young person went silent, and what

happened immediately after? Rupture in this model is often a behaviour that has been inadvertently reinforced — for example, a young person's refusal to speak that has been followed by extended attempts to persuade, which is itself aversive and which has taught the young person that the strategy works. The behavioural model's risk is that it misses the meaning of the rupture, and a young person whose withdrawal is a bid for a different kind of attention will not be helped by a contingency analysis alone.

**The humanistic and person-centred model.** Rupture is a failure of the relational conditions, and its repair is the restoration of those conditions: a return to accurate empathic understanding, to unconditional positive regard, to congruence. Ruth Be warned in 1963 that the counsellor is likely to become less accepting when the client becomes more difficult, and this "the counsellor has failed" model is both the most humane and the most demanding of the models, because it requires the practitioner to accept that the rupture is partly a failure of the practitioner's own conduct. The model's risk is that it can be used to absorb responsibility that the practitioner should be sharing with the young person, and that it can generate a great deal of self-reflection in place of actual change.

**The integrative and process model.** Rupture is a process event to be tracked, understood, and worked with collaboratively, drawing on all three of the above. The most useful formulation is that ruptures are inevitable, that their occurrence is a sign that the young person is engaged enough for the relationship to matter, that their content is the most valuable data available, and that the repair itself is often the most potent therapeutic event in the episode. The risk of the integrative approach is its very flexibility, which can become a way of avoiding commitment to any one understanding of what happened.

### 4.3 The mechanism of rupture

Ruptures have a common structure, which is worth teaching explicitly because it makes them recognisable in the moment. A rupture typically involves: a **disagreement** about what is happening or what should be done; a **disengagement** in which one party withdraws from the collaborative work; and a **displacement**, in which the affect belonging to the current relationship is transferred to a less threatening target — a symptom, a homework task, a complaint about the school, a demand for a different appointment time, or an attack on a third party.

The most useful single clinical rule in this area is the one that many trainees learn the hard way: **the displacement is the rupture**. When a young person begins talking angrily about their school in the middle of a session about their relationship with their mother, or becomes suddenly concerned about the format of the homework, the practitioner should treat the sudden shift as information about the relationship rather than as a topic. Asking, gently and with genuine curiosity, "it seems like something has just changed for you — what has happened in here?" is frequently more productive than following the topic.

## 5. Repair: Restoring the Collaborative Frame

Repair is a skill, and it is a learnable one. The components of an effective repair are as follows.

**Notice.** The practitioner must recognise the rupture, which is harder than it sounds, because ruptures are frequently covert and because the practitioner's own defensiveness is activated by them. The most common missed ruptures are the young person's boredom, the young person's apparent agreement that is in fact compliance, and the young person's deferential silence.

**Name it.** The rupture is named, in plain language, without blame, and with the invitation of the young person to correct the practitioner's account. "I think something has just happened between us, and I'm not sure what it was. Have I got something wrong?" The naming must be offered, not imposed. A practitioner who declares "we are in a rupture" to an adolescent is performing a technical concept rather than making a human observation, and it will be received as such.

**Take responsibility for the practitioner's part.** This is the most counter-intuitive and most effective element. A rupture in which the practitioner acknowledges some contribution to the difficulty, however small, is substantially more likely to be repaired than a rupture in which the young person is invited to explain what is wrong with them. "I pushed you on that question and I think I pushed too hard" is a sentence with a very high yield.

**Explore the meaning.** Once the frame is restored, the content of the rupture is explored, and this is the moment at which the most valuable material in a case often appears.

**Return to the task.** Repair is completed by returning to the work, and the return should be explicit: "where were we?"

### 5.1 The failed repair

Not every rupture is repaired, and the practitioner must know what failure looks like in order to respond to it. Signs of a failed repair include: the young person leaves the session in a worse state than they arrived; the topic is avoided for several sessions; the young person begins cancelling; the rupture is never named; or the practitioner returns to the same behaviour that produced the rupture. The response to a failed repair is not more technique. It is a change in the plan, a consultation, and sometimes a change of therapist — which is not a failure of the young person or of the practitioner but an honest recognition that the fit is wrong, and one that a young person is entitled to.

## 6. Consent, Collaboration, and Feedback

Three further elements of the relationship require attention, all of them contemporary developments in the field and all of them with particular force in work with young people.

**Informed consent as a relational process.** Chapter 54 treats the ethical and legal architecture of consent and Appendix A provides the model documents. The point relevant here is that consent is not a form signed at the beginning; it is an ongoing, revisable, and developmentally appropriate process in which the young person's understanding and agreement are actively sought and periodically revisited. A young person who signs an assent form without understanding what will happen has not consented to anything, and the practitioner who treats the signature as the end of the matter has converted a form into a substitute for a conversation.

**Collaborative goal setting.** The literature is clear that the alliance is stronger and outcomes better when goals are collaboratively constructed rather than prescribed. This is more difficult with young people, who may not have the language to state a goal, and it is more necessary, because a goal the young person does not own will not be worked on. Practical techniques include scaling, the miracle question, the "what would you like to be different" question, and the deliberate construction of goals in the young person's own words. Chapter 35 develops the solution-focused approach and Chapter 33 the work with parents and adolescents.

**Routine outcome and alliance feedback.** The contemporary standard is that practitioners should routinely ask about and provide feedback on progress, and that the feedback should be structured rather than informal. The Session Rating Scale and the Working Alliance Inventory in their short forms can be administered in two minutes at the end of a session and reviewed every four to six sessions. This is not a bureaucratic imposition; it is a way of making the rupture visible before it becomes a dropout.

## Further Reading for This Chapter

- Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. *Journal of Counseling Psychology, 4*(2), 95–107. — The classic statement of the person-centred conditions; read it as a theory of the relationship rather than as a technique.
- Beauchamp, T. L., & Brehm, J. T. (2015). *Therapeutic relationships: Theory, research, and practice*. Guilford. — The standard contemporary treatment of alliance theory, rupture, and repair, including the rupture research.
- Safran, J. D., & Muran, J. C. (2006). *The therapeutic relationship*. — A concise and clinically oriented treatment with a strong research base.
- Wampold, B. E. (2015). How important are the common factors in psychotherapy? *World Psychiatry, 14*(3), 270–277.
- Hill, C. E., & Lambert, W. M. (Eds.). (2013). *Therapeutic alliance in the context of cross-cultural and culturally diverse psychotherapy*. — Essential for the developmental and cross-cultural material.

## 7. The Relationship Across the Theoretical Traditions

| Dimension | Humanistic | Cognitive | Behavioural | Systemic/Relational | Constructivist |
|---|---|---|---|---|---|
| The relationship is... | the mechanism of change | a necessary condition for collaboration | a behaviour subject to contingency | a parallel process reflecting the wider system | a negotiated co-construction |
| The practitioner's stance | congruent, unconditional, empathic | collaborative expert, teacher | coach and contingency manager | consultant to the pattern | collaborator, co-author of the account |
| The client is... | the expert on their own experience | a rational agent whose appraisals can be examined | an organism whose behaviour can be reinforced | a position in a system | the author of a story under revision |
| The main relational risk | absorption of the young person into the practitioner's warmth | over-argument and persuasion toward a belief the practitioner prefers | control, and the loss of the young person's say | overriding the individual's wish for the system's good | appropriation of the client's voice |

| Dimension | Humanistic | Cognitive | Behavioural | Systemic/Relational | Constructivist |
|---|---|---|---|---|---|
| Alliance failure looks like | the practitioner becoming less accepting as the client becomes harder | the client complying verbally while privately disengaging | the client complying for the reward while the underlying function goes unaddressed | the client becoming "the good one" in the room to protect the system | the practitioner steering the narrative to a preferred conclusion |
| Repair is achieved by... | returning to empathy, acceptance, and congruence | re-establishing shared curiosity and returning to the data | changing the contingency and returning to the task | naming the pattern in the room, including the practitioner's part | asking whose account is being told and who has been left out |
| What ismost important developmentally | safety, being taken seriously, continuity of the adult in the young person's life | developmentally appropriate verbal and written processing; concrete, near-term goals | a developmentally appropriate task analysis; reinforcement that the young person values and can obtain | explicit inclusion of the parents and the school; developmentally appropriate boundaries | language the young person can use; no imposition of an adult's categories |

## 8. Repair in Practice: A Sequence

The following sequence is offered as a usable clinical procedure. It is drawn from the integrative literature and it is generic across orientations, though the interpretation of the rupture's content will be orientation-specific.

**Step 1: Attend to your own state.** Before attending to the young person, notice your own arousal, your own defensiveness, and your own pull towards a particular action — to explain, to reassure, to press, to change the subject, to end the session early. This step is easy to omit and it is the step that most improves the quality of what follows. A practitioner who is flooded cannot empathise, and a practitioner who is defensive cannot hear.

**Step 2: Verify the discrepancy.** Compare what the young person is doing with what they were doing earlier or elsewhere in the session, and with what they told you last week. The discrepancy is the data: not the behaviour itself, but the change in the behaviour.

**Step 3: Name it tentatively.** Offer the observation as a hypothesis, in the young person's terms, with an invitation to correction. "You've been very quiet since we started talking about your dad. Is that right? Have I got something wrong?" The offer must be genuinely open. A practitioner who has already decided that the young person is "in a rupture" will be heard to be making an announcement.

**Step 4: Take your share.** If there is any part of the rupture to which your conduct contributed — you pushed, you were late, you disagreed sharply, you went somewhere they did not want to go, you were distracted, you spoke over them — say so, briefly, without apology that requires the young person to reassure you. "I think I pushed too hard on that. I'm sorry. I'd rather you tell me if it's too much."

**Step 5: Explore.** Only now, with the frame restored, go to the content. The most important question at this point is not "what are you feeling?" but "what happened in here?" — because the rupture is in here, and the young person may have learned more about the relationship in the last three minutes than in the previous six

sessions.

**Step 6: Repair the practical as well as the emotional.** If the rupture concerned the time of day, the length of the session, the presence of the parent in the waiting room, the homework, the school form, or anything else concrete, fix it. A young person who has been disappointed by the practitioner on a practical matter will not proceed to the emotional repair until the practical repair has occurred, and the practical repair is frequently the one that matters more.

**Step 7: Return explicitly.** "Where were we?" and then, ideally, a lighter agenda than the one that was interrupted.

**Step 8: Review the pattern.** After the session, write down what happened, your interpretation, and what you will do differently. If the same rupture recurs in the same form across three or four sessions, the interpretation is probably wrong, and the case needs consultation.

## 9. The Relationship in Family, Group, and Systemic Work

The therapeutic relationship in work with a family or a group is not one relationship but a network of them, and the network is the object of the work.

**In family work.** The practitioner is in a room with the people whose patterns are being examined, and the practitioner's conduct is itself part of the system. A practitioner who is warmly supportive of the mother and cool with the adolescent has, in the language of the systemic tradition, taken a side, and has enacted the very triangulation the family brought to the room. The practical implication is that a family practitioner must be able to notice their own positioning, and must be willing to repair it in front of the family, because a repair enacted in the room is a demonstration of what repair looks like. The parallel-process material that becomes available when a family member treats the practitioner as a confidant is among the most valuable data in family work, and it is also a hazard, because the practitioner who becomes the confidant of one member has changed the system in the act of observing it.

**In group work.** The group is a miniature social world, and the relationship between the leader and the members is replicated in the relationships among the members. A group member who is silent in the group and animated at the break is giving a group-formulation practitioner exactly the information required. Group work also provides a unique relational opportunity: a young person who cannot tolerate intimacy with an adult can sometimes tolerate it with a peer, and the observation of that tolerance is itself a form of assessment and a form of intervention. Chapters 55 and 56 of the curriculum map address group work with young people, and the common factors in group are discussed in Chapter 6.

**In consultation.** In school consultation the practitioner is in a relationship with a teacher, and the alliance is not with the student at all. The literature on consultation is consistent that the outcome of consultation is determined principally by the consultation relationship and the consultee's perception of the consultant's competence and credibility, and that consultation is frequently delivered as advice-giving under another name, which reliably produces resistance. A consultant who asks what the teacher is already trying and what has been tried will be received very differently from one who arrives with a plan. Chapter 56 develops this material.

**In telehealth.** The medium affects the relationship in specific, examinable ways: loss of the subtle embodied cues that constitute much of empathic accuracy; the young person's awareness that the

practitioner is in a separate room with a computer; the presence of family members within earshot; the absence of a waiting room; and, for some young people, the relative safety of a mediated space. The literature on teletherapy alliance is smaller than the literature on in-person alliance but broadly consistent, and the recommendation that the practitioner attend explicitly to the medium — asking what the room looks like, whether the young person is alone, whether they can speak freely — is one of the more useful practical findings. Chapter 55 develops this in detail.

## 10. Case Illustration: The Girl Who Stopped Talking

**The referral.** Priya, fifteen, is referred by her school for "non-engagement" after a period of attendance at 80 per cent and a fall to 55 per cent over six weeks. She has not refused to attend; she attends and then sits in the corridor outside the counselling office for the entire session. The school counsellor, who has seen her for four sessions, reports: "She'll say 'fine' if you ask how she's doing. Doesn't talk. Ate lunch in the library this week with a book, so she's engaging with something. Parent in for the first session and did all the talking."

**The background.** Priya lives with her mother, her father, and a younger brother of eleven. She is in the second year of a new school following a house move in year ten. Her father is a doctor; her mother left paid work to care for the children and has recently begun a training course. She was previously a competitive swimmer and gave it up in the summer. There is no history of mental health treatment. The mother reports that "she's fine, she's just quiet, she was always quiet."

**The first session.** The practitioner, working in a school-based service, arranges to see Priya in a small room rather than in the counselling office, and to see her without the parent present. Priya comes, and sits, and says very little. After a few minutes she says: "Is this going to be like the other one?" The practitioner asks what she means. Priya says: "Are you going to ask me about my feelings?" And then: "Because she did, and I just said everything was fine, and then I had to sit there for forty minutes."

**The analysis.**

The first rupture in this case occurred before the practitioner arrived, in the previous service, and it is a rupture of the specific kind that the previous practitioner could not have identified because the young person had not spoken. A client who asks a question like "are you going to ask me about my feelings?" is performing a displacement: the topic under discussion is the previous sessions, and the affect belonging to that relationship is being carried into a complaint about the new practitioner's likely method.

The second rupture is occurring in the present session, in the form of a defensive silence that reproduces the structure of the previous one. The practitioner who responds to the silence with a series of open, feeling-oriented questions — "How are you feeling right now?", "What is that like?" — will reproduce the previous service's failure exactly. The likely mechanism is that Priya has learned that feeling questions in this setting lead to a demand that she produce a feeling account, which she is unwilling and unable to supply, and which she experienced as being disbelieved when she did supply a minimal one.

**The repair.** The practitioner's next move is the sequence of Section 8 applied in the present, and it is not subtle:

> *"No, I don't want to do that. I think that question is a trap and I'd rather we didn't. I want to know how you spend your time — what you do, who you talk to, what the days look like. That's the kind of thing I'm interested in. And if I ask you something and you'd rather I didn't, tell me*

*and I'll stop."*

This move does several things at once. It answers the question directly, which is what Priya asked. It names the trap and thereby validates her previous experience. It offers a different structure, which addresses the learning from the previous service. It asserts the practitioner's own view (the feeling question is a trap) rather than merely accommodating, which protects the practitioner from a psychodynamic reading in which the silence is itself the symptom. And it gives Priya a second method for managing the interaction — she can say "not that one" — which is a form of the agency that the previous service did not offer.

**The working alliance.** The next several sessions should be spent on the working alliance rather than on the presenting problem, and the goal should be the explicit and practical one of establishing predictability: the same time, the same room, the same ten minutes at the start. The outcome measure should be the Session Rating Scale, which asks the young person to rate the session on four dimensions, and which — as the practitioner should tell her — is a tool she can use if the sessions are not working. Making the feedback instrument available to the client is a specific, and here highly indicated, application of the feedback principle.

**The rupture that will come.** Somewhere in the fifth to eighth session, a rupture will occur, and its most likely content is the father. The case material makes it a reasonable prediction that the withdrawal began in earnest after the father's schedule changed to include more evening work and the mother began her training course, and that the school move and the end of swimming are coincident rather than causal. The rupture will probably arrive as a complaint about the practitioner — that she is a waste of time, that she does not understand, that the sessions are pointless — or as an abrupt cancellation. This is the moment the sequence is for. The practitioner should not retreat, and should not interpret the father's schedule as the content. The content of the rupture is the relationship; the father's schedule is the material from which the young person is building the current interaction.

**The systemic caution.** The mother's presence in the first session and the report that "she was always quiet" are both significant, and the systemic reading — that Priya's silence is a function of a family in which one member has become the spokesperson for the whole — should be held in mind and should be examined, but not now. Attempting a systemic intervention in the first three sessions with a young person who has already experienced one service that did not listen to her is a second violation, and a more damaging one. The relationship comes first.

**The ethical considerations.** Three issues are engaged. The first is confidentiality: Priya will need to know precisely what is shared with the school, and the practical answer in a school-based service is that information is shared with the school only with her agreement, and that the practitioner is required to pass on information where there is a risk of serious harm. The second is the limits of the school's request: the school has asked for a "non-engagement" intervention, and the practitioner's obligation is to Priya's interests, not to the school's administrative convenience. The third is the question of whether the previous service's approach constitutes a failure of care that warrants a complaint, and whether the practitioner has any obligation to raise it. The professional answer is that the practitioner has a duty to the young person in the room, and that the duty includes not repeating the failure, but that there is generally no duty to initiate an inquiry into a previous service unless the young person or a third party has raised a safeguarding concern.

## 11. Evidence and Research Support

**The alliance–outcome relationship is among the best established findings in the field.** The strength of the evidence is unusual and has survived three decades of methodological criticism, including the demonstration that the correlation is not entirely attributable to a third variable.

**Rupture is common and its management is consequential.** The research literature indicates that ruptures occur in a substantial majority of episodes, that they are more frequent in the early sessions, that they are more common in longer treatments, and that successful resolution is associated with better outcome. The finding that the experience of rupture and repair can itself be therapeutic — that a rupture successfully worked through deepens the alliance and is remembered by clients as a turning point — is consistent across the psychodynamic, integrative, and person-centred literatures.

**Specific techniques add to the relationship.** Component studies and dismantling studies indicate that specific procedures account for outcomes over and above the relationship in conditions for which they are indicated. The dismantling of the components of behavioural and cognitive treatments for anxiety, depression, and conduct problems has been extensive, and in each case the removal of the specific component reduces the outcome for that component's target. The implication is that the relationship is necessary and not sufficient.

**Feedback improves outcomes.** The literature on routine outcome monitoring in routine services is unusually consistent: practitioners who receive structured feedback on their clients' progress produce better outcomes than those who do not, and the effect is larger when the feedback is provided systematically and repeatedly than when it is provided once. The effect is present in the adult literature, is smaller but detectable in the adolescent literature, and is generally larger in services where staff are less experienced.

**The limits of the evidence.** Much of the alliance research has been conducted with adults, with self-report measures that are vulnerable to social desirability, and with a focus on the alliance as a unitary construct rather than on its components, whose contributions may differ. And the developmental literature on the alliance with young people, while growing, remains thinner than the adult literature, which means that some of the specific claims made in this chapter about adolescents are extrapolations supported by developmental theory rather than by direct evidence.

## 12. Strengths and Limitations of a Relationship-Centered Account

**Strengths.** It identifies the condition under which all other work becomes possible. It gives the practitioner a diagnostic instrument that is usable in the moment and does not require a formal assessment. It integrates across theoretical orientations, so that a practitioner who changes orientation does not lose the core skill. It is teachable in a way that most clinical concepts are not. And it supplies a mechanism for continuous learning, because the rupture carries information about the work as it is actually being experienced.

**Limitations.** The relationship is not directly manipulable: the practitioner cannot simply decide to relate well, and the literature gives no reliable manual for producing a good alliance. The relationship can be over-valued, producing a practice in which everything is negotiated and nothing is challenged. The emphasis on the relationship can obscure the need for structure, for challenge, and for the discomfort that some young people need in order to change. And the research base is dominated by adult samples, so the specific claims about adolescents are less well established than the field's confident tone suggests.

## 13. Cultural and Ethical Considerations

**Culture and the alliance.** The alliance is not a universal. Studies of the therapeutic alliance across cultures have found that the alliance–outcome relationship holds in many cultural settings, but that the *expression* of a good alliance varies, and that some of the components Western instruments treat as essential are less uniformly valued across cultures. In settings with strong family or collective orientation, the young person's expectation may be that the practitioner will speak to the family rather than to the young person; the Western assumption of individual confidentiality can itself be a rupture. And where mistrust of institutions is a well-founded response to the young person's experience — with the mental health system, with schools, with authorities — the practitioner begins the work already behind. Chapter 48 addresses the power differential in detail, and Chapter 49 addresses the specific dynamics with LGBTQ+ youth.

**The ethical asymmetry of work with minors.** Three features of the relationship with a young person make it ethically unlike the relationship with an adult. The young person's consent is complicated by the legal requirement of guardian permission, which produces a situation in which a person who cannot lawfully consent to the therapy is nonetheless in the room and is being asked to be honest in it. The practitioner holds more power than in adult work, and the power is institutional as well as personal. And the young person is more likely to be harmed by a breach, and less likely to have the resources to report one.

**Three obligations follow.** First, the practitioner must be scrupulous about confidentiality and must explain, concretely and in advance, what will and will not be shared, with whom, and why. A young person who does not know the answer to "who else knows what I say?" cannot give meaningful consent to anything. Second, the practitioner must not allow the therapeutic relationship to become the only relationship in the young person's life, and must actively work toward the young person's connections with peers, with family, and with the ordinary institutions of their daily life. Third, the practitioner must anticipate the exploitation risk and manage the setting accordingly: no closed doors, no touch, no sessions in the practitioner's car, no private electronic messaging channels, no gifts, and a clear statement to the young person and the guardian about the boundaries that apply.

**The last of these is worth a further word.** A young person who has been harmed by an adult will often test the boundary, and the test may be subtle and may be a bid for contact rather than an attempt at exploitation. The practitioner's response should be warm, unpanicked, and absolutely clear, and the practitioner should not be discouraged by the discomfort of delivering it.

## 14. Key Terms

**Key Terms at a Glance**

- **Therapeutic alliance** — The collaborative and affective bond between practitioner and client, including the shared commitment to the work; the construct most consistently associated with outcome.
- **Working alliance** — The task-focused component of the therapeutic relationship: the shared agreement about the goals and the division of labour.
- **Rapport** — Interpersonal ease and mutual liking; the most superficial component of the relationship and the one most often mistaken for the alliance.
- **Therapeutic bond** — The affective component of the relationship: the sense of a genuine, caring, and reliable connection.
- **Necessary condition** — A factor without which the mechanism of change cannot operate; the relation of the alliance to technique.
- **Rupture** — A breakdown in the working alliance, overt or covert, in which the collaborative frame is interrupted.
- **Covert rupture** — A rupture expressed through lateness, minimal responding, withdrawal, or other indirect behaviour rather than through open conflict.
- **Repair** — The process by which a ruptured collaborative frame is restored.
- **Displacement** — The transfer of affect belonging to a relationship onto a less threatening target; in the clinical setting, the sign that a rupture is occurring.
- **Parallel process** — The systemic hypothesis that the therapeutic relationship enacts the client's other relationships; the observation that what happens in the room reflects what happens elsewhere.
- **Enactment** — The repetition, in the therapeutic relationship, of a relational pattern formed elsewhere.
- **Working-through** — The repeated, exploratory processing of a relational pattern across sessions with the aim of modifying it.
- **In-session rupture** — A rupture occurring in the current session, as distinct from one that has led to a cancellation, an absence, or a termination.
- **Session Rating Scale** — A brief four-item measure of session quality, including overall alliance, used for routine feedback; typically completed in under two minutes.
- **Working Alliance Inventory** — A widely used self-report measure of the working alliance with subscales for goal, task, and bond.

**Key Terms at a Glance (continued)**

- **Client Outcome Questionnaire** — A brief measure of session-level progress, developed within the client-centred tradition and used in outcome monitoring.
- **Structured feedback** — The systematic provision of a client's ratings to the practitioner, shown to improve outcomes in routine services.
- **Collaborative goal setting** — The practice of constructing goals jointly, associated with stronger alliance and better outcome.
- **Interpersonal rupture** — A rupture in which the young person withdraws, complies superficially, or becomes hostile.
- **Boundary management** — The practices by which a practitioner protects a young person from exploitation in the therapeutic setting.

## 15. Chapter Summary

The therapeutic relationship is a necessary condition of change and the medium through which every theory's technique operates. Its components are distinguishable: the working alliance, the therapeutic alliance, rapport, and the therapeutic bond, and the confusion of rapport with alliance is a common clinical error.

The alliance–outcome relationship is one of the most robust findings in the field, with meta-analytic correlations in the region of 0.25 to 0.30 in adult samples and smaller but detectable effects in adolescent samples. The relationship is partly causal and partly a marker of practitioner skill and of the client's circumstances, and the effect is greater for outcomes in the domain of the therapeutic focus. Specific technique contributes over and above the relationship, which is why the relationship is a necessary condition and not a sufficient cause.

Rupture is an ordinary, expectable feature of the work, and its content is the most valuable information available about the young person's experience of the therapy. Four models of rupture — psychodynamic, behavioural, humanistic, and integrative — offer different and partially competing accounts, and the differences have clinical consequences. Repair is a learnable sequence: notice, name tentatively, take your share, explore, repair the practical, return to the task, review the pattern.

The relationship with a young person is not the adult relationship. It is constrained by developmental limits on verbal and metacognitive processing, by institutional power asymmetry, by the young person's shorter time horizon, by the developmental instability of the identity the alliance is formed with, by the presence of adults who are not the client, and by the requirement that the practitioner's behaviour be consistent with what the young person has been told by other adults.

## 16. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the distinction between the alliance and rapport, and give a clinical example of a young person with high rapport and a poor working alliance.
2. State the logical argument for the relationship being a necessary condition of change, and state the strongest objection to it.
3. Summarise what is known about the direction of causation in the alliance–outcome relationship. What would it take to establish the direction experimentally, and why has that not been done?
4. Describe the developmental constraints that the alliance literature places on working with adolescents, and identify which of them you think is most often overlooked.
5. Compare the psychodynamic, behavioural, and humanistic models of rupture. What does each one make possible, and what does each risk missing?
6. What is the displacement, and why is the rule "the displacement is the rupture" clinically useful?
7. Work through the repair sequence in Section 8 as it would apply to a young person who becomes angry and says "this is stupid" and wants to leave.
8. Why is it a mistake to treat a signed assent form as the completion of informed consent?
9. Identify three specific risks that attend the therapeutic relationship in work with minors, and state the practice that addresses each.
10. In what ways does the alliance in family work differ from the alliance in individual work, and what does the practitioner have to do differently?

## 17. Practice Activities

**Practice and Application**

### Activity 5.1 — The Rupture Video (2 hours, individual, then triad)

Record a 12-minute mock session in which a student plays a fourteen-year-old who has "nothing to say." Play it back with a peer who takes the role of observer, using the Section 8 sequence as the observation framework. The observer records: the moment the rupture became recognisable; the practitioner's first response; whether the practitioner named it; whether the practitioner took a share; whether the repair was completed; and where the exchange was lost. Rotate roles so that each student has practised both the practitioner and the young person. This is the single most useful activity in the first year of the course, and the reason is that the rupture is invisible to the practitioner in the moment and visible on tape.

### Activity 5.2 — Session Rating Scale Administration (60 minutes, paired)

Practise administering the four Session Rating Scale items to a peer, covering all four dimensions and the final global item, in under two minutes, in a way that a fifteen-year-old would find unremarkable. Then administer the same items to the peer as if the peer were a distressed adolescent, and note what changes in your delivery. Finally, have the peer rate the session and discuss the discrepancy between your intention and their rating. The last step is the whole exercise: the gap between intention and experience is the rupture.

### Activity 5.3 — The Parallel Process Journal (3 hours, ongoing, 10 hours, reviewed)

For a period of ten days, keep a brief evening journal. Each entry records one interaction in which you were the more powerful party, and answers three questions: what did I want? what did I assume the other person wanted? what did the other person actually seem to want? Read the journal at the end. This exercise develops the capacity to notice one's own contribution to relational difficulty, which is the foundation of the repair sequence and the foundation of the self-examination required in Chapter 48.

## 18. Annotated Reading Pathways

**For the theory of the relationship.** Read Rogers' 1957 paper as a theory rather than as a technique, and read the work of the relational and intersubjective theorists alongside it. Then read the contemporary integrative accounts, which specify what the relationship must contain for different populations.

**For the empirical literature.** Read a meta-analysis of the alliance–outcome relationship in adults, then the corresponding adolescent literature, then a study of the directionality question. Note what the adolescent literature does and does not establish; the answer to the second question is more limited than the confident practice of the field suggests.

**For rupture and repair.** Read the Safran and Muran treatment, and then the research literature on the association between rupture resolution and outcome. Then read the developmental and cross-cultural material on rupture with young people, which is thin and growing.

**For the ethical material.** Read the sections of the professional codes that bear on boundaries, on the limits of confidentiality, and on work with minors, and then read the child welfare literature on the specific risks in therapeutic settings with young people. The codes tell you what is required; the child welfare literature tells you why the requirements are as specific as they are.

## Practice Laboratory and Extended Worked Example

This laboratory turns **The Therapeutic Relationship: Alliance, Rupture, and Repair** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the common substrate on which all theory-specific methods rest. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Sofia is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Sofia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Sofia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Sofia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Sofia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Sofia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for The Therapeutic Relationship: Alliance, Rupture, and Repair

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **The Therapeutic Relationship: Alliance, Rupture, and Repair** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **The Therapeutic Relationship: Alliance, Rupture, and Repair** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

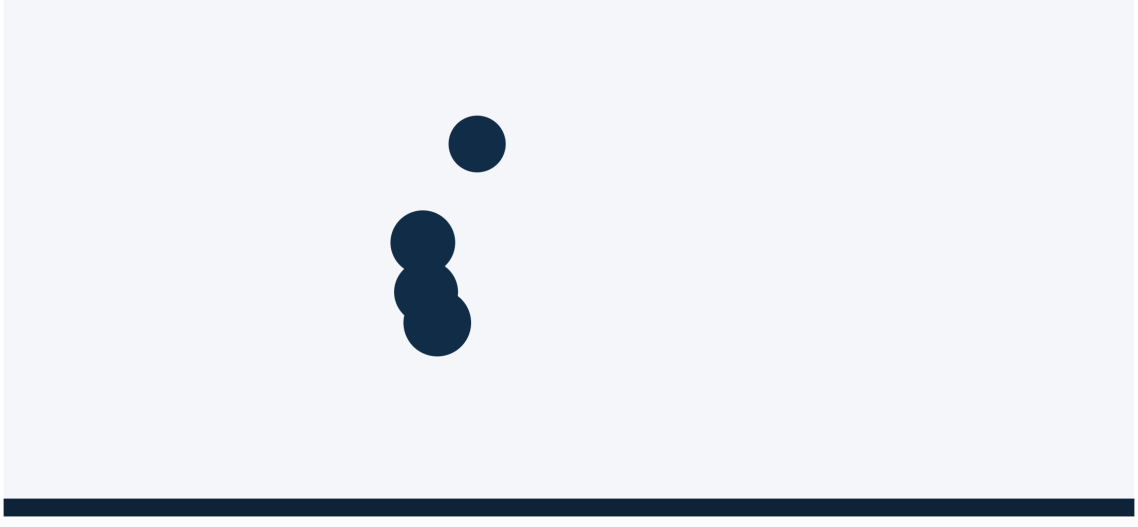
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **The Therapeutic Relationship: Alliance, Rupture, and Repair**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain The Therapeutic Relationship: Alliance, Rupture, and Repair to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 6

# Common Factors, Placebo Effects, and Specific Factors

What all therapies share, what is unique to each, and why the distinction matters for ethical practice.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain what the common factors tradition is, what it asks, and where it came from.
- Define the placebo effect, the placebo component of treatment, and the structural conditions of placebo responding, and distinguish them from one another.
- Describe the Destre-Barnum effect and explain what it does and does not establish about specific techniques.
- Summarise the principal common factors and the evidence for each.
- Evaluate the "all therapies are equivalent" conclusion and state the four arguments against it.
- Explain the methodological problems that have limited the common factors research programme.
- State what the common factors literature implies for a practitioner working with children and adolescents, and what it does not imply.
- Distinguish the "common factors as necessary" claim from the "common factors as sufficient" claim, and describe the position the evidence supports.

## Chapter Overview

The common factors question is the most consequential methodological question in the history of counseling. Put at its simplest: *if two very different therapies produce similar outcomes for similar patients, is the specific technique doing the work, or is the common setting, the common relationship, and the common expectation doing the work?* The answer determines what a graduate student should study, how a practitioner should think about method, and what the profession can honestly claim to the public.

The question has been asked since the 1930s and it has been answered in four successive ways, each of which has been misunderstood in turn. The first answer, offered by the psychodynamic and humanistic traditions, was that the specific technique is what matters, and that the common factors are a secondary set of conditions necessary for any technique to work. The second answer, offered by the behaviourist tradition, was that the specific technique is what matters and that the common factors are largely artefacts of the research design — the expectation of improvement, the attention, and the contact were all products of the treatment condition. The third answer, from the common factors movement, was that the common factors account for most of the variance in outcome, and that the specific factors contribute relatively little. The fourth and current answer, which is where the evidence and the argument now stand, is more careful than any of its predecessors: the common factors are necessary, they are not sufficient, the specific factors contribute meaningfully where they are indicated, and the size of the specific contribution is much smaller than the proponents of any single technique have claimed.

This chapter traces that history, examines the placebo literature and the Destre-Barnum effect, sets out the common factors and their evidential support, and draws the conclusions that follow for practice and for theory. It is one of the most important chapters in the book, because the student who misreads it will either lose faith in technique or lose faith in method, and both errors are disastrous in different ways.

## 1. The Question the Common Factors Tradition Asks

The common factors tradition is a research programme, not a theory of therapy. It asks a single question:

> *Given that comparable problems treated by comparable therapists with comparable levels of competence in comparable settings produce comparable outcomes across a wide range of therapeutic methods, what is doing the work?*

The formulation above contains the key terms, and each of them is a term of art.

**Comparable problems.** The comparisons are made between patients with similar presentations. This is a substantial restriction, and it is the point at which the tradition's conclusions are most often over-extended. The finding is not that a cognitive therapy and a psychodynamic therapy produce identical outcomes for a child with panic disorder and a child with conduct disorder. It is that, within a defined population, the difference in outcomes between well-conducted therapies of different types is small relative to the difference between a well-conducted therapy and no treatment at all.

**Comparable therapists.** The therapists are of comparable training, competence, and experience. The finding does not extend to the difference between a competent and an incompetent therapist, which is large, and which the pragmatic trial literature shows to be one of the largest contributors to outcome variability.

**Comparable settings.** The comparison is made within a setting, not across one. The finding does not extend to comparing a well-resourced specialist service with an under-resourced general service, which is another large effect.

**Comparable outcomes.** The outcome measures are usually standardised symptom scales and occasionally functional measures, rather than the client's own appraisal of change. This is a further restriction with direct implications, discussed in Section 8, because the Client Outcome Questionnaire — the measure that operationalises the humanistic criterion — typically shows larger differences between therapies than do the standard symptom scales.

**The size of the difference.** The central empirical finding, stated conservatively, is that the differences in average outcome between bona fide therapies are small relative to the differences between therapy and no therapy, and that the differences shrink still further when the quality of the outcome measure is taken into account.

## 2. The History of the Question

### 2.1 The therapeutic equivalence finding and its reception

The finding emerged most clearly from the work of the psychotherapy research groups in the 1970s. The most influential single study was the 1979 research project of the National Institute of Mental Health, which compared cognitive therapy and imipramine in the treatment of depression, and which was followed in 1986 by a smaller comparative trial of cognitive therapy and interpersonal therapy. The result — no significant difference in outcome between the two psychotherapies, and no significant difference between the two psychotherapies and the medication condition — was widely reported in the popular press as the demonstration that psychotherapy and medication are interchangeable and that all psychotherapies are equivalent. Neither conclusion was supported by the data. The trial compared two treatments known to work with a third treatment known to work, in a population selected for a favourable response; it did not include a no-treatment condition, and the "equivalent" treatments were both active.

The finding that therapies produce similar outcomes was, in fact, a finding about the **efficacy of two good treatments**, and its interpretation required an experimental control that the field did not have: a condition designed to control for the common elements while omitting the specific element. That control — the "equivalence trial" design, in which a credible treatment is compared with a credible treatment that deliberately omits the technique while retaining the relationship, the expectation, and the contact — was developed by the common factors researchers themselves, and it is the design on which the field's conclusions about specific factors rest.

### 2.2 The four positions

The historical positions can be arranged in a spectrum.

**Position 1: the specific factors are the active ingredients.** This is the position of the psychodynamic, behaviourist, cognitive, and systemic traditions in their strong forms. It is supported by the component studies: the dismantling of behavioural and cognitive treatments for anxiety and depression reliably reduces outcomes for the component's target, and the addition of a specific component to a treatment reliably improves outcomes for that target. The position is undermined by the equivalence finding and by the observation that unorthodox methods produce outcomes comparable to orthodox ones.

**Position 2: the common factors are the active ingredients.** This is the position developed most fully by the common factors movement, and it is supported by the equivalence findings, by the Destre-Barnum effect, and by the consistency of the alliance–outcome relationship across methods. It is undermined by the component studies, by the fact that the equivalence findings come from equivalence trials of equivalent methods, and by the fact that the common factors are not specific enough to guide practice.

**Position 3: the common factors are necessary but not sufficient.** This is the position the contemporary evidence supports. On this account, the common factors — the relationship, the expectation, the attention, the rationale — constitute the conditions under which any specific technique can operate, and the specific technique determines what the change consists of and whether it occurs at all.

**Position 4: the question is misconceived.** This is the position that the common factors and specific factors are not separable, and that the useful unit of analysis is the therapy as a whole. It has some force, and it has a certain explanatory appeal, but it is not a position from which a practitioner can act, and it has the effect of making the research programme impossible to complete.

## 3. The Placebo Effect and the Placebo Component of Treatment

### 3.1 Distinguishing the terms

Three things are routinely confused, and the confusion has damaged the field's thinking for four decades.

**The placebo effect** is a psychological and physiological response to the *expectation* of improvement, and it can be produced by inert treatment, by attention, and by the context of treatment. It is a real phenomenon, it is measurable, and it is part of almost every treatment outcome, including surgical ones.

**The placebo component of treatment** is a feature of the *design* of a treatment rather than of the person's response. As classically formulated by Kenneth Rice and elaborated in the psychotherapy literature, a treatment has a placebo component to the extent that it offers a credible rationale, a coherent explanation of

the difficulty, an attention from a credible practitioner, and an expectation of improvement. The extent of the placebo component is a matter of design, not of the client's suggestibility.

**The placebo condition** in a research design is the control condition against which a specific treatment is tested. It is not a neutral condition; it is a treatment with the specific component removed. The methodological error of the field's early trials was to use "no treatment" and "wait-list" controls, which omit the placebo component, and then to attribute the whole difference to the specific technique.

### 3.2 Why this matters for the common factors question

The logical structure is as follows. When a trial finds that two treatments produce similar outcomes, the finding is consistent with the conclusion that the shared elements are responsible — but only if the two treatments actually share their active ingredients, and only if the trial controls adequately for the common elements. In trials comparing an orthodox therapy with a deliberately technique-deficient "attention control", the attention control is designed to replicate the relationship, the expectation, and the attention, and to omit the technique. Where the two conditions produce comparable outcomes, the technique appears to contribute little. Where the specific-technique condition produces superior outcomes — which is what the component studies show for well-indicated conditions — the technique contributes meaningfully.

### 3.3 The nocebo and the adverse effects of expectation

Expectations can also operate in the negative direction, and the nocebo effect is relevant to practice in two ways. First, a practitioner who introduces a treatment with an over-optimistic framing — "this will work" — creates a comparison in which the young person's experience of a difficult therapy session becomes evidence that the treatment is failing. The literature on expectancy discrepancies in therapy suggests that a substantial minority of clients experience such discrepancies and that they predict poorer outcome and earlier termination. Second, the nocebo effect is relevant to the physical symptoms that accompany anxiety and somatisation, and a young person who has been told for years that their chest pain is "all in their head" has been given a nocebo frame.

## 4. The Destre-Barnum Effect

### 4.1 The original study

In 1964, David Destre and Robert Barnum published a study of college students who received, in a group setting, either a detailed explanation of the efficacy of a test of "how much one could improve by listening to a tape recording of one's own voice" or a placebo lecture on "an equally plausible but factually erroneous account". The group receiving the detailed and accurate explanation improved significantly on the test; the group receiving the placebo account did not.

The finding is usually described as showing that "it is the explanation, not the method, that produces improvement", and it is regularly cited as a decisive argument for the common factors. That description is inaccurate, and the inaccuracy is instructive.

### 4.2 What the study actually showed

The study showed that **credibility of explanation affects outcome**. It did not show that specific technique is irrelevant, because the design confounds at least three variables: the credibility of the explanation, the

quality of the expectancy induced, and the nature of the specific task. The "true" condition involved a specific and repeated practice — listening to one's own recorded voice — and the "placebo" condition did not. The study cannot distinguish between "the explanation was better" and "the activity was better" and "the expectation was better".

### 4.3 What the study has been used to do

The Destre-Barnum study has been used to support the strong common factors position, and it is the kind of use that gives the common factors movement a bad name. A practitioner who cites it to justify not learning technique, or to justify offering any approach with a good rationale, has misunderstood it. The more defensible reading is that **credibility of explanation is a variable worth optimising**, which is a real and actionable finding for a practitioner designing a treatment rationale for a young person and family, and which supports the humanistic and the systemic traditions' long emphasis on the coherence of the frame.

### 4.4 The methodological critique

The broader category of study in which Destre-Barnum sits — the comparison of a "true" treatment with a "placebo" treatment that differs in the provision of a rationale — has been comprehensively critiqued. The critique is that such comparisons confound the specific technique with the expectancy, the attention, and the credibility of the rationale, and that they therefore cannot isolate the technique. The standard resolution is the "attention control" or "credible control" design, in which both conditions receive a rationale and a comparable amount of attention and the specific technique is the only systematic difference. The field's methodological position has improved considerably since the 1970s, and the Destre-Barnum study now stands in the literature mainly as an example of a design that does not support the conclusion for which it is cited.

## 5. The Principal Common Factors

The common factors tradition has identified a set of elements that appear across all recognised therapies. The list varies by author, but the following six are usually included.

[Figure placeholder — Common factors in counseling practice, showing the six elements that recur across therapeutic traditions.]

**The therapeutic alliance and its components.** The quality of the relationship, the shared agreement on goals, the reciprocity of responsibility, and the breaking of impasse. This is the common factor with the strongest empirical support, and it is the subject of Chapter 5.

**Empathy and understanding.** The practitioner's accurate grasp of the young person's experience, communicated in a way the young person can recognise as accurate. The evidence base is substantial, and the most robust finding is the paired one: accurate empathy predicts better outcome, and *inaccurate* empathy — empathy that the practitioner believes is accurate but is not — predicts worse outcome. The therapeutic relationship literature has recently turned its attention to practitioner accuracy as a skill in its own right, with promising results in the training literature.

**Positive expectation and expectancy management.** The client's expectation of improvement is one of the best single predictors of outcome, and the practitioner is a major source of that expectation. The practical implications are considerable: the framing of the treatment, the explanation of the mechanism, the

normalisation of the difficulty, and the explicit discussion of what will happen if the treatment does not work are all expectancy-management interventions.

**A coherent rationale or conceptual frame.** The theory that explains to the young person what is happening and what will help. The common factors research suggests that the credibility of the frame matters, and the case for providing a clear explanation of the process of therapy is now made in most of the evidence-based treatment manuals. A young person who has been told what will happen, in terms they can understand, is a young person who is more likely to experience setbacks as part of the process rather than as evidence of failure.

**Ritual and procedure.** The consistent structure of sessions, the setting, the time, the agenda, the homework, and the recurring shape of the meeting. The ritual provides predictability, which is a precondition for disclosure, and it also provides the young person with a structure within which to manage their own anxiety. The literature on the value of structure and predictability for young people is consistent.

**Attention and the allocation of time.** The sheer quantity of professional attention devoted to the young person is a component of every treatment, and the common factors research suggests that it is more consequential than is generally acknowledged.

## 6. Specific Factors: The Evidence That Technique Matters

The specific factors position rests on three kinds of evidence, and it is worth stating them in detail because they are what make the current position a "necessary but not sufficient" position rather than a common factors position.

### 6.1 Component studies

A **component study** examines a treatment in which one of its components has been removed, with everything else held constant. The classic series removed components from behavioural and cognitive treatments for anxiety, depression, and conduct problems. The results are consistent: removing the specific component produced a significant reduction in outcome for the target of that component, and removing components not targeted at a particular outcome did not.

The most instructive single example is the dismantling of the **coping cat** in the treatment of snake phobia: the group that received the standard behavioural treatment with the fear hierarchy improved more than the group that received the same treatment without the hierarchy, but both improved more than the group that received the desensitisation without the homework. The component mattered; the homework mattered; the relationship was constant across all three groups.

### 6.2 Additive studies

An **additive study** adds a specific component to a treatment that is otherwise identical. The best-known example is the addition of a homework component to a standard treatment for depression, which produced a measurable improvement in outcome in some studies. The general finding is that the addition of an appropriate specific component improves outcomes for the component's target, and that the improvement is largest when the component is delivered competently and when the client engages with it.

### 6.3 The differential efficacy of matched and mismatched treatments

The third strand is the finding that a treatment which is well matched to a specific condition outperforms a treatment which is not. The clearest examples are the following: the behavioural and cognitive-behavioural treatments of anxiety, which are among the best-supported interventions in the entire field and are clearly superior to supportive or non-directive approaches for specific anxiety presentations in young people; the multisystemic and functional family therapies for antisocial behaviour, which outperform individual treatment; the exposure-based and trauma-focused treatments for post-traumatic symptoms, which outperform nonspecific treatment; and the family-based approaches to eating disorders, which outperform individual approaches in the cases where family involvement is possible.

The conclusion from this strand is precise and important: **there is no general hierarchy of therapies, but there are specific indications for which therapy is appropriate for which condition.** A student who has learned that is in a position to make a real clinical decision.

## 7. The Methodological Problems

The common factors research programme has been limited by four methodological problems, and a student who is evaluating a claim about the common factors should look for how a given study has handled each.

**The problem of the conditions compared.** The finding of equivalence holds for well-conducted therapies of comparable quality. It does not extend to comparing a well-conducted therapy with a poorly conducted one, with a non-equivalent modality, or with a treatment delivered by practitioners at very different levels of skill.

**The problem of the outcome measure.** Standardised symptom scales systematically understate the differences between therapies, partly because they measure the presence of symptoms rather than the person's experience of change. Where the client is asked how much they have improved, the differences between therapies are larger. The field's standard practice of relying on symptom scales was adopted for good methodological reasons — reliability, comparability, and resistance to expectancy bias in the client — but it has the effect of systematically favouring the common factors position.

**The problem of statistical power and of equivalence testing.** Studies designed to detect differences between therapies are usually underpowered to detect small differences, and a failure to reject the null hypothesis is not evidence of equivalence. The methodological standard now requires equivalence or non-inferiority designs with pre-specified margins, and the number of studies using them remains small.

**The problem of the researcher's allegiance.** The common factors movement has had a strong prior commitment to its conclusion, and the field's tendency to treat the equivalence finding as decisive was, in part, a movement's tendency to find its conclusion in the data. The alliance researchers have been the most careful in this respect, and the current, more careful position owes a great deal to that care.

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## Further Reading for This Chapter

- Frank, J. L. (2015). *Persuasion and healing* (3rd ed.). Johns Hopkins University Press. — The fullest scholarly treatment of the common factors question, with a critical analysis of the placebo literature and

a fair account of the theoretical consequences.

- Wampold, B. E. (2015). How important are the common factors in psychotherapy? *World Psychiatry, 14*(3), 270–277. — A short, direct, and well-argued statement of the current position from within the field.
- Kazdin, A. E. (2010). *Psychotherapy for children and adolescents*. — Contains the developmental and methodological material on component studies with young people.
- Society for the Psychological Study of Social Issues (2011). *Consensus statement on the evidence base for the effectiveness of psychotherapy*. — A careful consensus document that states the current position with appropriate precision.
- Hollon, S. D., & Cox, M. (2001). *Concepts and procedures in the therapy of depression*. — An example of the component-study literature at the heart of the specific-factors position.

## 8. The Alliance as the Principal Common Factor

Chapter 5 examined the alliance in clinical detail. Here the question is narrower: what is the alliance's status in the common factors argument?

The position the evidence supports is this. The alliance is **the common factor that mediates the operation of the specific factors**. The mechanism runs as follows: a young person must trust the practitioner sufficiently to disclose; a specific technique such as exposure or thought recording requires disclosure and collaboration to be implemented at all; the technique therefore cannot operate without the relationship; and the relationship is consequently the pathway through which the specific factor reaches the young person. On this account, the alliance and the specific factors are not competitors. They are a condition and a content.

This claim is supported by a specific and rather striking body of evidence. Studies that have examined the alliance in the context of a specific treatment, using process models, have found that the alliance-outcome relationship is mediated by the young person's engagement with the specific technique: the alliance predicts outcome to the extent that it predicts whether the client actually does the work of the therapy. A young person who is well allied but does not complete the exposure work does not improve. This is a finding of considerable practical significance, because it locates the practitioner's most valuable intervention precisely: not in doing the technique better, but in securing the young person's collaboration with the technique.

It also explains the otherwise puzzling finding that the alliance–outcome relationship is stronger in the early sessions than in the later ones. The early sessions are where the practitioner has the greatest leverage over the young person's willingness to engage with the work, and the early alliance is the best available predictor of whether the specific factors will be delivered at all.

There is one important qualification, which is that the claim is strongest where the specific technique is one that requires active participation, which is most of the techniques in this book. It is weaker in the case of treatments whose efficacy is largely a function of the exposure itself — some pharmacological conditions, and the behavioural interventions in which a change in the environment does most of the work.

## 9. Other Common Factors in Practice

### 9.1 Expectancy management

Expectancy management is the practice of shaping, honestly and explicitly, the young person's expectation of what the process will be like and what it will produce. It has a number of concrete components, all of which are teachable.

**Normalising the difficulty.** A young person who has been told that the first few sessions will feel strange, and that they may leave feeling worse, will interpret that experience differently from a young person who has not. The practice is honest and it is empirically supported.

**Attributing discomfort to the process.** Where a treatment involves deliberate discomfort — exposure, behavioural activation, confrontation of a cognitive distortion, a family session in which a difficult exchange is required — the discomfort should be attributed in advance to the process rather than to the young person's pathology. The practitioner who says "this is going to be hard and it is supposed to be" prevents the young person from concluding that they are getting worse.

**Attributing improvement to the process and to the young person.** A young person who improves is told what they did that made the difference, because the attribution is accurate and because it strengthens the young person's sense of agency and self-efficacy. Chapter 45 treats self-efficacy in detail.

**Handling expectancy discrepancies.** Where the young person's expectations and the experience diverge, the discrepancy should be addressed directly rather than left to fester. The evidence suggests that unaddressed discrepancies predict poorer outcome and earlier termination.

## 9.2 The rationale

Every treatment in this book has a rationale that can be stated to a young person in a sentence or two. The statement should contain: what the practitioner believes is going on; what will be attempted; what the young person will be asked to do; what the expected course is; and what the young person should do if it is not working. The rationale is a common factor because it serves the expectancy, because it provides the cognitive scaffolding that the young person will need in order to make sense of their own experience, and because its credibility affects the young person's willingness to engage.

A rationale must also be honest about the limits of the treatment. Over-promising is a common and consequential error, and the practitioner who has promised more than the treatment can deliver has set up an expectancy discrepancy in the first session.

## 9.3 Ritual and structure

The ritual of the session is a common factor, and it is particularly important with young people. A consistent time, a consistent place, a consistent agenda shape, a consistent ending with a brief summary and a small task, and a consistent check-in at the start all provide the predictability that disclosure requires. The structure should be introduced explicitly and explained, so that the young person knows that the structure is a deliberate contribution to their safety rather than an arbitrary formality.

## 9.4 Attention and dosage

The literature on the relation between the amount of professional attention and outcome is less developed than the alliance literature, but the general finding is that more contact is generally better, up to a point, with a plateau in the middle of a treatment and a decline in the tail. In practice, this supports the following: the early sessions should be relatively frequent; the middle of the treatment should not be allowed to drift; and

the ending should be planned rather than accidental.

## 10. What the Common Factors Literature Implies for Theory

The common factors literature has three implications for the practising theorist, and they are frequently stated in a way that the evidence does not support.

### 10.1 The relationship is not a substitute for technique

The strong claim — that the common factors account for most of the variance in outcome, and that specific factors therefore barely matter — is not supported. The component studies, the additive studies, and the differential efficacy findings all point in the other direction. The accurate claim is that the specific factors account for a substantial proportion of the variance that is left after the common factors are accounted for, and that the size of that proportion varies by condition and by population. For panic disorder in a young person with good engagement, the specific component matters a great deal. For a young person with mild adjustment difficulty and a good alliance, it may matter less, and the difference is not a reason to withhold a technique but a reason to calibrate the intensity of the plan.

### 10.2 Every theory must specify the conditions of the relationship

A theory that does not specify what the practitioner must do in the relationship is incomplete. This is a real and important claim, and it is the reason that the humanistic tradition's account of the conditions of growth remains the most fully developed account in the field. The cognitive theories specify the collaborative stance; the behavioural theories specify the reinforcing behaviour of the practitioner; the systemic theories specify the practitioner's positioning and the repair of positioning ruptures; the third-wave approaches specify the therapist's behaviour in the functional contextual sense. A graduate student should be able to state, for each theory, what it requires of the practitioner in the relationship — and should note that this is one of the most useful criteria for choosing among them for work with young people.

### 10.3 Theories should be judged partly on their rationale

A theory's explanatory power is a legitimate criterion for evaluation, but so is the quality of the rationale it offers a young person and a family. The cognitive account of panic is better than most alternatives at explaining to a fifteen-year-old why her body is doing what it is. The behavioural account of avoidance is better than most alternatives at explaining why avoidance is so hard to give up. The attachment account is better at explaining why a child is difficult to reach. The quality of a theory's account of the young person's experience is a clinical fact, and it is a legitimate reason to prefer it.

## 11. The Common Factors Literature and Work with Young People

The common factors research is overwhelmingly adult-based, and the developmental adjustments required for young people are not minor. Four of them are worth stating in summary.

**The relationship is more heavily weighted.** With a young person, the relational component carries more of the load, because the young person's ability to engage with a technique depends on developmental capacity as well as on trust. A cognitive technique that an adult can execute alone may require an adult to co-execute it with a fourteen-year-old, and that requirement changes the relationship.

**The rationale must be developmentally simplified without being distorted.** The behavioural account of a young person's tantrum can be explained to a child; the cognitive account of a conditional negative thought can be explained to a fourteen-year-old; neither can be explained in its full form to a seven-year-old, and the developmentally appropriate adaptation is not simplification but re-expression at the level of the child's own concepts.

**The expectancy of the young person may be the dominant expectancy.** Where the young person has no expectation of improvement — and this is common with young people who have been in services before and have learned that nothing works — the expectancy-management work is the primary intervention, and it may be more important than the technique.

**The cultural and institutional context of the alliance is more complex.** The young person's expectation of an adult authority figure may be shaped by their experience of the adults in their life, including their parents, their teachers, and the authorities who have previously made decisions about them. The practitioner is entering a field of existing relationships and must establish credibility within it, which is a different and more demanding task than the adult equivalent.

## 12. Case Illustration: A Therapist Who Believed the Common Factors

**The situation.** A 26-year-old therapist, two years post-qualification, has become a committed adherent of the common factors position. He has read Frank and Wampold and has concluded that technique is largely irrelevant. He works in an agency that specialises in adolescent substance use, where the evidence-based protocols are structured, manualised cognitive-behavioural programmes, and where the agency's contract requires the use of the protocol and the reporting of outcome measures.

**What he does.** He drops the protocol. He conducts unstructured sessions in which he talks with the young person about their life, offers reflections, and trusts the relationship to do the work. He is warm, perceptive, and well liked by the young people, and the feedback forms the agency collects show high ratings for the sessions.

**What happens.** The outcome data are bad. Over the following nine months his caseload shows a markedly lower rate of goal attainment than the rest of the team, and the agency's clinical manager, who is also a good clinician, begins to raise the issue. Two of his young people are readmitted to a higher level of care. He concludes that the agency is a "medicalised" service that has been captured by the pharmaceutical-industrial complex, and that the measurement of outcomes is a tool of institutional control. He brings this view to supervision, where he is listened to respectfully and then asked some questions.

**What the questions reveal.** The questions in supervision are not about whether the common factors matter. They are:

- What do you believe is happening with each of the young people on your caseload?
- If you cannot answer that in terms that the young person would recognise, how will you know whether anything is changing?

- What is your plan if a young person tells you they are going to use again this weekend?
- What is the plan for the young person who has been discharged once already this year?
- What are you going to do about the drink-driving risk in the case of the sixteen-year-old whose father drives her home?

These questions expose the position's cost. The common factors position, in its strong form, offers no answer to questions about technique, because it holds that technique does not matter; and the practitioner who adopts it discovers that he must either abandon the position or answer these questions in a way that is unguided. The second option is more dangerous than the first, because the questions get answered by improvisation.

**The resolution.** The resolution is not that the common factors are unimportant. It is that the therapist had converted a *necessary condition* into a *sufficient one*, and had converted a research finding about comparative effectiveness into a clinical decision about method. Under supervision, he returns to the protocol as the *scaffolding* of the work rather than as its entirety: he uses the protocol's session structure, its motivational and cognitive preparation, its behavioural contracting, and its outcome monitoring, and he keeps the relational stance that he genuinely does well and that the data show his patients value. He also has to say, in supervision and in his own reflection, that his earlier claim that outcome measurement is a tool of institutional control was a rationalisation for a clinical failure he had not wanted to confront, and that the data had been available to him all along.

This case is worth holding on to because the therapist's reasoning is not foolish, and the position he holds is genuinely held by serious thinkers. The failure is not intellectual; it is a failure of the discipline described in Chapter 1: a theory held in a way that could not be disconfirmed by the experience of the practitioner, and the substitution of an attractive general position for the disciplined, provisional commitment that professional practice requires.

## 13. Evidence and Research Support

**Well supported.** The alliance–outcome relationship across therapies and populations; the mediation of technique by the alliance; the contribution of specific techniques for specific conditions, as shown by component and additive studies; the effect of structured feedback on outcome; the importance of expectancy; the value of the alliance in predicting early engagement; the general finding that bona fide therapies differ little from one another in average outcome while differing considerably in their appropriateness for particular problems.

**Supported with qualifications.** The claim that common factors account for a large share of outcome variance — true as a general statement, but the size of the share varies, and the calculation depends on the outcome measure used. The claim that a credible rationale affects outcome — supported, but the classical Destre-Barnum design does not support it in the form in which it is usually cited. The claim that the placebo component explains the whole of the placebo response — a substantial part of it, but not all, and the design of the field's early trials overstated the finding.

**Not supported.** The claim that all therapies are equivalent in any respect that would interest a clinician. The claim that technique does not matter. The claim that the equivalence finding licenses offering any method with a good rationale. The claim that measuring outcomes is a distortion of clinical practice rather than a condition of it.

**Methodologically weak.** The common factors literature is dominated by adult samples, by self-report alliance measures, and by studies with modest samples and moderate design quality. Its conclusions are more secure for adults than for young people, and the field's confident generalisation to adolescents is not fully warranted by its evidence.

## 14. Strengths and Limitations of the Common Factors Position

**Strengths.** It has done the profession a great service in correcting the extravagant claims of technique partisans. It has produced a rigorous research literature on the alliance and on the process of therapy. It has grounded the profession in the ordinary conditions of practice rather than in the special conditions of the trial. And it has given the practitioner a set of transferable, teachable skills that work across orientations.

**Limitations.** It has, in its strong form, encouraged a therapeutic minimalism that leaves practitioners without guidance in the situations that matter most. It has a strong prior commitment that has shaped the interpretation of data in some of the classic studies. It has tended to under-emphasise the conditions — particularly with young people, and particularly with those who are not motivated — under which technique is the thing that changes the outcome. And its alliance research has focused on the measurement of alliance rather than on the training of practitioners in producing it.

## 15. Cultural and Ethical Considerations

**The cultural location of the common factors.** The "therapeutic alliance" as it is measured in the Western literature assumes a dyadic, voluntary, verbal, individual, and reasonably equal relationship. In settings where the young person's expectation is that the practitioner will be an agent of the family or the community rather than an ally of the individual, the alliance as operationalised may be a poor description of what is happening, and a practitioner who optimises it may be inadvertently undermining the young person's ties to their family, which in some cultural contexts is the primary source of resilience. Chapter 48 addresses the power differential that underlies this problem.

**The ethical significance of expectancy management.** Expectancy management is an ethical activity and it has a limit. The limit is that the expectancy must be honestly managed rather than manufactured. A practitioner who tells a young person that a treatment will work when the evidence for it in the young person's case is weak has not managed the expectancy; they have deceived the young person in order to secure compliance, and they have done so to a person who is entitled to an accurate account of the options. The young person is owed a truthful account of what is known, what is uncertain, and what will be monitored — and the more honest the account, the more effective the expectancy management, because the young person's own hope is a stronger source of expectancy than the practitioner's reassurance.

**The ethical significance of the measurement critique.** The argument that outcome measurement is an instrument of institutional control is sometimes true and is often used to avoid accountability. The correct response is to insist on the distinction between measurement as surveillance and measurement as care, and to note that the instruments of the field are among the few mechanisms by which a young person's own appraisal of their progress enters the clinical record. A service that does not ask the young person how they are doing is not thereby more authentic; it is simply less informed.

# 16. Key Terms

**Key Terms at a Glance**

- **Common factors** — The elements shared across all recognised therapies, thought to account for a substantial share of outcome variance; the subject of a research programme rather than a theory of therapy.
- **Specific factors** — The components of a treatment unique to its theory, claimed by their proponents to be the active ingredients of change.
- **Placebo effect** — The psychological and physiological response to the expectation of improvement, independent of the specific content of the intervention.
- **Placebo component of treatment** — The extent to which a treatment's design supplies a credible rationale, attention, and expectancy of improvement.
- **Placebo control** — A control condition that is designed to reproduce the common elements of treatment while omitting the specific technique; the correct comparator for testing a technique.
- **Wait-list control** — A control condition with no treatment elements at all; an inadequate comparator for attributing outcome to a specific technique.
- **Equivalence trial** — A design comparing two bona fide treatments of comparable quality, used to establish that specific techniques contribute little.
- **Component study** — A design in which one component of a treatment is removed with all else held constant, testing whether that component is necessary.
- **Additive study** — A design in which a component is added to a treatment, testing whether it improves outcome.
- **Destre-Barnum effect** — The 1964 finding that a more credible explanation of a procedure produced better results than a less credible one; widely cited for a conclusion the design does not support.
- **Expectancy discrepancy** — The gap between a client's expected and experienced outcome, which predicts poorer outcome and earlier termination when unaddressed.
- **Nocebo effect** — The counterpart of the placebo effect, in which the expectation of harm or of negative symptoms increases the likelihood of those symptoms.
- **Mediation** — The mechanism by which a common factor such as the alliance enables the delivery of a specific factor; the basis of the claim that the alliance is the pathway for technique.
- **Mediational model** — A model in which the relationship between process and outcome is explained by a third variable, such as engagement with the technique.
- **Bona fide therapy** — A treatment with a credible theoretical rationale, delivered by a competent practitioner, and having been shown to produce improvement in at least one controlled study.

**Key Terms at a Glance (continued)**

- **Necessary but not sufficient** — The current position on the common factors: the shared elements enable change but do not constitute it.
- **Therapeutic minimalism** — The practice of withholding specific technique on the grounds that the relationship is sufficient; a clinical and ethical risk, and the subject of the case in Section 12.
- **Qualitative inquiry bias** — The systematic tendency of therapy research to rely on standard symptom scales, which understate differences between therapies by measuring symptoms rather than the person's experience of change.

## 17. Chapter Summary

The common factors tradition asks what produces improvement when comparable problems, comparable therapists, comparable settings, and comparable outcome measures are used across therapies that differ in method. The central finding is that the differences in average outcome between well-conducted therapies of different types are small relative to the difference between therapy and no treatment, and small relative to the differences produced by therapist competence and by the quality of the service.

The placebo literature shows that a treatment has a design-level placebo component — a credible rationale, attention, and expectancy — and that early trials using wait-list controls overstated the contribution of technique. The Destre-Barnum study is frequently cited for a conclusion it does not support; it demonstrates that credibility of rationale matters, which is a real and actionable finding but a much narrower claim.

The current position, supported by component studies, additive studies, and the differential efficacy of matched treatments, is that the common factors are **necessary but not sufficient**: they are the conditions under which a specific technique can operate, and the alliance is the principal pathway through which the technique reaches the young person. This reconciles the common factors and specific factors positions rather than choosing between them.

The common factors literature is overwhelmingly adult-based, and its developmental implications for young people are substantial. And the strong version of the position, in which technique is dispensed with on theoretical grounds, is a therapeutic minimalism that leaves the practitioner without guidance in exactly the situations that require guidance most.

## 18. Review and Discussion Questions

**Review and Discussion Questions**

1. State the common factors question in your own words, and list the four restrictions on the equivalence finding that make it weaker than it is usually taken to be.
2. Distinguish the placebo effect, the placebo component of treatment, and the placebo control condition, and explain why the choice of control condition in the 1970s trials overstated the role of technique.
3. What did the Destre-Barnum study actually demonstrate, and how does that differ from the conclusion usually drawn from it?
4. Explain what a component study is and what it has consistently shown about specific techniques.
5. Why is the use of standard symptom scales a systematic bias in the common factors argument?
6. In what sense is the alliance the "principal common factor", and what is the mechanism by which it enables the specific factors?
7. What is an expectancy discrepancy, how does it arise, and what does the evidence say about its consequences?
8. Summarise the case in Section 12. What was the therapist's reasoning error, and what did supervision reveal?
9. State the evidence-based position on the common and specific factors in three sentences, and then state the strongest objection to that position.
10. What does the common factors literature imply about the minimum ethical standard of care for a young person with mild difficulties and a good relationship with the practitioner? What does it not permit?

## 19. Practice Activities

**Practice and Application**

### Activity 6.1 — The Rationale Workshop (2 hours, groups of four)

Each group takes a treatment from this book — behavioural activation, exposure with a cognitive preparation, a family meeting, a stabilisation protocol — and writes the rationale that would be given to a thirteen-year-old and to the young person's parent. The rationale must contain: what is happening; what will be attempted; what the young person will be asked to do; what the expected course is; what happens if it does not work. Each group then delivers the rationale to a partner playing a suspicious fourteen-year-old, and the partner's job is to object. The group revises. This exercise is harder than it looks and it is one of the most useful in the course, because a rationale that cannot survive a fourteen-year-old's objection is not yet a rationale.

### Activity 6.2 — An Expectancy Discrepancy in Your Own Work (90 minutes, individual)

Take a client from your placement or a peer role-play. Write out what the young person probably expected when they first came, what they probably expected at the third session, and what they are likely to experience. Identify the point at which the discrepancy is most likely to arise, and write the two sentences you would use to address it. Then practise saying them aloud, and ask a partner whether they sound like an explanation or like a reassurance. The distinction matters, and it is hard to hear in one's own speech.

### Activity 6.3 — The Minimalism Temptation (60 minutes, individual, written)

Write a page in which you argue, as persuasively as you can, that technique is unnecessary and that a good relationship is sufficient. Then write half a page identifying the three cases in your current work for which the argument would be most dangerous and why. This exercise is a form of inoculation: a student who has argued the position in the abstract is far less likely to be captured by it in the room.

## 20. Annotated Reading Pathways

**For the history and the argument.** Read Frank (2015) in full if you can, and then read Wampold (2015) as a corrective. Frank's treatment of the placebo literature is the most intellectually careful thing in the field, and it is also the most sceptical of the common factors conclusion.

**For the component and additive literature.** Read one component study in full, with attention to its design, and then read its replication. The best-known series are those for phobias, for depression, and for panic, and the depression series is the most instructive because it includes the homework component.

**For the developmental material.** Read the adolescent alliance literature in Chapter 5 and its sources, and the sections of the developmental and child psychotherapy texts on the alliance with young people. This is thin literature, and the thinness is itself a finding about the state of the field.

**For the ethical questions.** Read the arguments for and against outcome measurement in practice, and consider the position of a young person in a service that measures and the position of a young person in a service that does not. The codes of the profession require both, and the tension is real.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Common Factors, Placebo Effects, and Specific Factors** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is what all therapies share, what is unique

to each, and why the distinction matters for ethical practice. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Daniel is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Daniel's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Daniel explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Daniel chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Daniel has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Daniel can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Common Factors, Placebo Effects, and Specific Factors

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Common Factors, Placebo Effects, and Specific Factors** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Common Factors, Placebo Effects, and Specific Factors** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

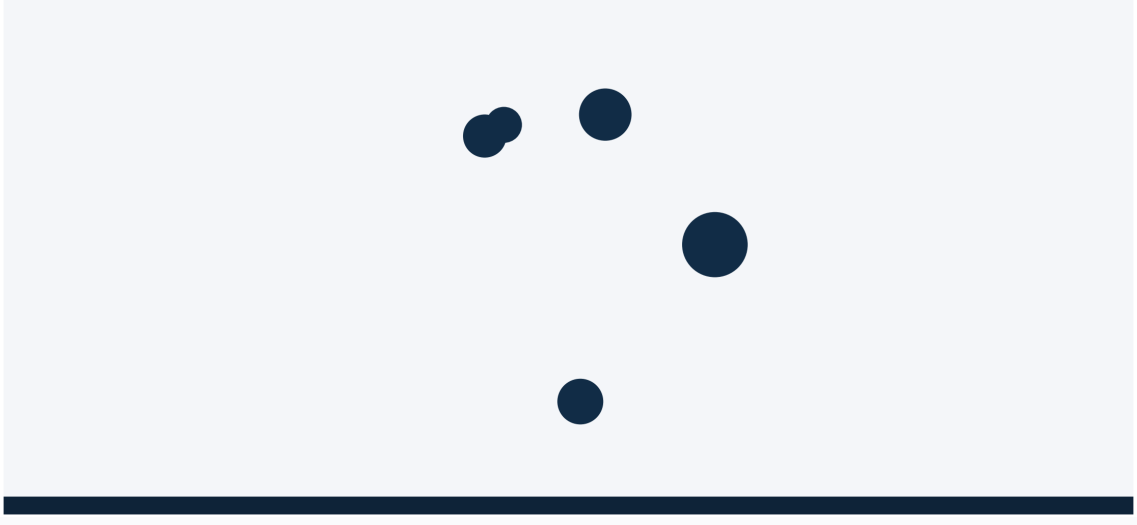
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Common Factors, Placebo Effects, and Specific Factors**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Common Factors, Placebo Effects, and Specific Factors to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 7

# The Evidence Base: Evaluating Counseling Theories

Outcome research, treatment manuals, and the appraisal of efficacy for youth-serving interventions.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain what "evidence-based" means in counseling, and describe how the term's meaning has changed since the 1990s.
- Describe the hierarchy of evidence and state where the various forms of counseling research sit within it.
- Explain the efficacy–effectiveness–implementation gap and illustrate each of the three stages.
- Describe the contribution and the limits of practice-based evidence.
- Critically appraise a systematic review or meta-analysis, including the questions of risk of bias, heterogeneity, publication bias, and ecological validity.
- Explain the specific problems that the counseling research literature presents for the evaluation of work with children and adolescents.
- State what a reasonably prudent practitioner should do with a body of evidence that is thin, contested, or internally inconsistent.
- Use measurement-based practice to make a defensible decision about continuing, modifying, or discontinuing a treatment.

## Chapter Overview

Every theory in this book makes a claim about what helps, and every student in this course will one day be asked by an employer, a regulator, a funder, or a family to explain why a particular intervention was chosen. The purpose of this chapter is to make that explanation possible, and to make it honest.

The chapter begins with the history of the evidence-based movement in counseling, because the term's meaning has shifted substantially and the older usage is still in circulation. It then sets out the hierarchy of evidence and locates the various forms of counseling research within it, and it explains the three-stage model of efficacy, effectiveness, and implementation that now organises the field's thinking about translation. It devotes considerable attention to the specific methodological problems of the counseling literature — the control condition, the outcome measure, the population, and the sampling — because a student who cannot identify these problems will over-trust the literature.

The second half of the chapter is practical. It provides an appraisal framework that can be applied to a systematic review in about forty minutes, a procedure for reading a meta-analysis, a worked case in which a treatment must be continued, modified, or discontinued on the evidence, and the ethical and cultural considerations that bear on the whole enterprise.

The central argument of the chapter is that the evidence base for counseling with young people is thinner, weaker, and more contested than the confident language of the field suggests, and that the correct response to that fact is not scepticism but method: careful appraisal, honest measurement, and a willingness to treat one's own practice as the place where the question is actually being answered.

# 1. The History of "Evidence-Based" in Counseling

### 1.1 The two meanings

The phrase "evidence-based practice" entered the counseling profession in the 1990s, imported from medicine, and it has been used with two distinct meanings that should not be confused.

**Evidence-based practice** means a practice in which the interventions used are those that have been shown, in controlled research, to be effective. This is the meaning most practitioners mean, and it is the meaning the profession's codes increasingly require. Under this meaning, the practitioner's obligation is to deliver interventions that have demonstrated support.

**Evidence-based practice** also, and in some literatures, means a practice in which the practitioner uses the best available research evidence *in conjunction with* clinical expertise and with the client's preferences and circumstances. This is the more demanding meaning, and it is the one the evidence-based practice frameworks in medicine and in the mental health field generally specify. Under this meaning, the research is a necessary input to a judgement, not the whole of the judgement.

The first meaning is tractable and administratively convenient. The second is the professional standard the codes intend, and it is the one that requires the framework of this chapter.

## 1.2 The empirically supported treatment movement

The movement for empirically supported treatments, which developed in the United States in the 1990s around the Division 12 of the American Psychological Association and the work of the Association for the Advancement of Behavioural and Cognitive Therapies, was a direct response to two problems. The first was that the field's research literature, while substantial, was not usable: a practitioner could not determine from the literature which interventions to offer, because the literature was large, heterogeneous, and inconsistently indexed. The second was that the field's status in the mental health marketplace was vulnerable, in a context in which third-party payers were increasingly requiring evidence of effectiveness.

The movement's central methodological contribution was the **empirically supported treatment criteria**: a treatment is a candidate for the empirically supported designation if it has been tested in at least two independent investigations, with a comparison group, with a specified population, with well-defined outcomes, and with a manual describing the treatment. A treatment that meets the criteria is a well-defined treatment; the designation does not assert superiority over other treatments, only that the treatment has been adequately studied.

The contribution is real and the movement should be credited with it. Its limitation is that the criteria are a claim about the state of the research, not a claim about the state of the practice, and the "empirically supported treatment" label came to be used, in both the profession and the payer community, as though it were the latter. A student who learns the criteria precisely will avoid a great deal of confusion.

## 1.3 The dissemination and implementation problem

The third phase of the evidence-based movement, from roughly 2010 onward, has focused on implementation: the recognition that a well-supported treatment is of little use if it does not reach the people who need it, and that the reach of evidence-based practice depends on organisational and system-level factors rather than on the persuasion of individual practitioners. This phase has been shaped by the recognition of the "learning health system", in which routine data collection and the routine comparison of outcomes are used to improve the service, and by the implementation science literature, which distinguishes between interventions that are effective in a trial and those that are effective in a routine service.

## 2. The Hierarchy of Evidence

The hierarchy of evidence, which descends from the most to the least internally valid:

1. **Systematic review and meta-analysis of randomised controlled trials.**
2. **Randomised controlled trial.**
3. **Quasi-experimental study with a control group and pre-post measurement.**
4. **Single-case experimental design.**
5. **Controlled cohort or case-control study.**
6. **Uncontrolled outcome studies and case series.**
7. **Expert opinion, consensus, and anecdote.**

Two cautions are required about this hierarchy. The first is that it ranks designs by internal validity, not by usefulness. A well-designed single-case experimental design in a routine service with a young person whose problem is not represented in any trial will be more useful to that young person than a meta-analysis of trials conducted with a different population. The second is that the hierarchy is a hierarchy of *single* studies; a single randomised trial, however well conducted, sits below a body of well-conducted observational evidence about a question that trials cannot answer.

Counseling research occupies an unusual position within this hierarchy, and a student should be able to state where each of the following sits and what each can answer.

| Design | Contribution | Typical limitation in counseling |
|---|---|---|
| Meta-analysis of RCTs | Pooled estimate of effect with confidence interval; comparison across treatments | Publication bias; heterogeneity of interventions; mostly adult outpatient samples |
| RCT | Causal inference about a specific intervention for a specified population | Small samples; manualised delivery; research-clinic conditions |
| Pragmatic / cohort-randomised RCT | Effectiveness in routine services | Fewer published; contamination between arms; practitioner variability |
| Quasi-experimental | Real-world effectiveness where randomisation is impossible | Confounding; baseline differences |
| Single-case experimental design | Individual-level causal inference with immediate feedback | Weak generalisation; reactivity; limited to a single individual |
| Controlled observational study | Association between process and outcome in practice | Selection effects; measurement confounding |
| Uncontrolled outcome study | Description of change over time | No causal inference; expectation effects; regression to the mean |
| Qualitative study | Meaning, process, and the client's own account of change | Not generalisable; analytic subjectivity |
| Practice-based evidence | Real-world effectiveness across a caseload | Case mix confounding; inconsistent measurement |

# 3. The Efficacy–Effectiveness–Implementation Gap

## 3.1 Efficacy

An intervention is **efficacious** if it produces better outcomes than a comparison condition under ideal conditions: trained practitioners, high adherence to protocol, a selected population, a research team invested in the study, and no competing demands on the practitioners' time. The trials that established the evidence base for the cognitive-behavioural therapies were overwhelmingly of this kind.

Efficacy is a real and important achievement, and the treatments that have demonstrated it should be used. But efficacy tells us about a treatment, not about a service.

## 3.2 Effectiveness

An intervention is **effective** if it produces better outcomes than it would have otherwise in the routine conditions of ordinary services. The difference between efficacy and effectiveness is a set of pragmatic factors, and they are substantial:

- **Practitioner competence and experience** vary in services in ways they do not in trials.
- **Case mix** in a service is more heterogeneous than in a trial, and the average trial participant is more likely to respond.
- **Time pressure** in a service is greater, and the treatment is delivered in fewer sessions.
- **Comorbidity** in a service is more common.
- **Engagement** is lower in a service, and drop-out is higher.
- **Fidelity** is lower in a service, because the demands of the setting compete with the demands of the protocol.

The pragmatic trial literature, which is the most important methodological development in the field since 2010, is the source of the most sobering findings in the profession: that average outcomes in routine services are considerably worse than in trials, that the variability between providers within the same model and the same protocol is large, and that a substantial proportion of the variance is attributable to the therapist rather than to the model.

## 3.3 Implementation

**Implementation** concerns whether an effective intervention is delivered, with fidelity, to the people who need it, in the settings where they are. The implementation literature makes a series of distinctions that are directly useful to a student planning a placement or a first post: whether the intervention is *feasible* in the setting, whether it is *adoptable* by the staff who would deliver it, whether it is *usable* in the workflow, and whether the organisation has the leadership, the data infrastructure, and the supervision to support it.

The practical implication is that the best intervention in the literature is frequently not the best choice in a particular service, and that the question "what is the evidence for?" is only the first of three questions. The other two are "what can this service actually deliver?" and "what would it take to deliver it better?" A student who asks all three will be more useful in a placement than a student who knows only the first.

# 4. Practice-Based Evidence

**Practice-based evidence** is generated by the systematic collection of outcome data in routine services and the analysis of that data with quasi-experimental methods, typically comparing outcomes for a provider or a team against a benchmark.

Its advantages are substantial. It describes what actually happens in the service, with the real clients, under real conditions, including the clients who would never have been in a trial. It is sensitive to practitioner variation, which is where much of the real variance in outcome lives. It is sensitive to the effect of the practice on the practitioner, because the data are available at the level of the caseload. And it is continuous, so a service can detect a deterioration in its outcomes long before an external evaluation would.

Its limitations are equally real. The comparison is not randomised, so case mix differences between providers can confound the comparison. Data quality depends on the discipline of the staff in collecting it, and it is often poor. Sample sizes per provider are small, so the confidence intervals around a provider's estimate are wide. And the data reflect the practice that was actually delivered, which is a confound as well as a strength: a provider who delivered a well-evidenced treatment poorly will have poor outcomes, and the data cannot tell you whether the model or the delivery was the problem.

The most useful way to think about practice-based evidence is as a **compass rather than a map**. It tells you which way things are going and whether to keep going, and it does not tell you the terrain.

## 5. The Population Problem

The most serious methodological problem in the counseling research literature, for a book written for students who will work with children and adolescents, is the population problem.

The overwhelming majority of the outcome research has been conducted with adult, outpatient, non-suicidal, largely white samples in the United States, Western Europe, and a small number of other countries. The specific features of the adolescent literature are:

**Smaller studies.** Adolescent studies are typically small, and are therefore underpowered to detect the differences between treatments that the equivalence finding is based on.

**Fewer component and dismantling studies.** The body of evidence that establishes that specific techniques work for specific problems is dominated by adult studies, so the translation to young people is an extrapolation.

**Developmental confounding.** An intervention that has been shown to work with adults may work with young people for a different reason, or may fail, and the adolescent studies are generally not designed to distinguish these possibilities.

**The attrition problem.** Drop-out from therapy is substantially higher with adolescents than with adults, and drop-out is not random: the young people who leave are the young people whose problems are worst, whose engagement was lowest, or whose families were least able to support the work. Any analysis that excludes dropouts — which is the majority of published analyses — therefore systematically overstates outcome.

**The family confound.** Many adult studies are conducted with individuals, and many adolescent studies are conducted with families or in school settings. The settings are not comparable, and the outcomes are not comparable, because the mechanisms differ.

**The consent confound.** Research with young people requires the cooperation of parents and, in some jurisdictions, of the school. The samples are consequently skewed toward families who are engaged enough

to consent, which is a systematically different population from the one the practitioner sees.

**The assessment confound.** The instruments used to measure adolescent symptoms were largely developed and normed on adult or on older youth samples, and the developmental appropriateness of a self-report measure for a nine-year-old is a substantive methodological question rather than a technicality.

The correct conclusion is not that the adolescent evidence is worthless. It is that the adolescent evidence base is **narrower than the adult evidence base, and that the field's habit of transferring adult findings to adolescents without comment is a form of unacknowledged extrapolation that a professional should make explicit.**

## 6. The Measurement Problem

The choice of outcome measure in a study is a consequential methodological decision, and the field's standard practice has a systematic bias.

**Symptom scales** such as the depression, anxiety, and conduct inventories used in most trials are reliable, comparable across studies, and resistant to expectancy bias in the client. They are also poor at capturing what matters most to the young person and to the family, and they are not the outcome that the client will care about. Their use systematically favours the common factors position, because a change in the client's own appraisal of their life is a change that specific techniques produce more reliably than the standard scales detect.

**Functional measures** — school attendance, educational achievement, the absence of disruptive behaviour, the presence of a peer relationship, the resumption of ordinary activities — are closer to what the client and the family want, and they are more often improved by specific techniques. They are, however, more vulnerable to the influence of factors outside the therapy, and they are harder to attribute.

**The client's own rating** of progress, captured by instruments such as the Client Outcome Questionnaire and the Session Rating Scale, is the measure most sensitive to the difference between therapies, and it is the measure that the profession's own humanistic tradition has always insisted upon. The practice of collecting it routinely has a second-order benefit, discussed in Chapter 53: it makes the young person's own appraisal visible to the practitioner.

**Observer-rated measures** — parent, teacher, and clinician ratings — are useful precisely because they are not the client's own account, and their disagreement with the client's own account is itself clinically informative. A young person who reports large improvement and a teacher who reports no change presents a formulation question, not a measurement problem.

## 7. The Control Condition Problem

This problem was introduced in Chapter 6 and is repeated here because it is the single most important methodological point for a student to carry into the literature. A trial that compares a specific treatment with a no-treatment or wait-list control cannot separate the specific technique from the common elements, and therefore cannot estimate the specific technique's contribution. A trial that compares the treatment with a credible control that retains the relationship, the attention, and the expectation is the design that can.

The practical consequence for the student is that when reading a claim of the form "treatment X is superior to treatment Y", the question to ask is what the comparison condition was. If it was no treatment, the claim

is about the package rather than the technique. If it was a credible alternative treatment, the claim is a genuine head-to-head comparison and is a much more valuable finding. And if it was a credible control condition designed to isolate the technique, the claim is the strongest available and should be read with corresponding care.

## 8. A Critical-Appraisal Framework

The following framework can be applied to a systematic review or meta-analysis in about forty minutes and is sufficient for the critical appraisal strand of this course.

### 8.1 The review question

What exactly was asked? A well-constructed review has a focused, answerable question that specifies the population, the intervention, the comparison, and the outcomes. A review with a vague question will have a heterogeneous answer, and heterogeneity in a meta-analysis is not a finding about the world but a finding about the review.

### 8.2 Search and selection

How was the literature searched, how was it screened, and how many studies were included from how many identified? The flow of studies through the review is the single most informative thing in it, and a flow diagram should be read before the results.

### 8.3 Risk of bias

What did the included studies do well and badly? The principal domains are randomisation, allocation concealment, blinding of assessors, attrition, selective reporting, and, in the psychotherapy literature, fidelity to the protocol. The question to ask is not whether the studies were "good" but whether the flaws are likely to inflate the effect.

The two bias issues of greatest concern are **attrition bias**, in which the dropouts are systematically different from the completers, and **selective reporting**, in which only the outcomes that reached significance are published.

### 8.4 Heterogeneity

Do the studies agree? A meta-analysis that pools studies with a statistical test of homogeneity and finds them heterogeneous should not be reporting a single pooled estimate without exploring the sources of the difference. A very high $I^2$ statistic means that the pooled number is an average of things that are not the same, and the clinically useful information is in the strata, not in the average.

### 8.5 Publication bias

How many studies are missing? The standard diagnostics — the funnel plot, the fail-safe N, and the trim-and-fill adjustment — are crude, and the field's estimates of the proportion of unpublished studies in psychotherapy research run high. A meta-analysis with a small number of studies should be read with particular caution, since the diagnostics are least reliable in that case.

### 8.6 Effect size and clinical importance

Is the effect large enough to matter? An effect expressed as a standardised mean difference of 0.2 is small, and its clinical importance depends entirely on the context — on the natural course of the problem, on the severity at baseline, and on the consequences of not treating. The field's tendency to report statistical significance without reporting clinical significance is a serious weakness.

### 8.7 Ecological validity

Would this evidence apply to my case? This is the most important question and the one most often skipped. Ask it explicitly: was the sample young? Were the participants similar to the young person I am seeing? Was the setting similar to my setting? Was the therapist similar to me? Was the treatment manualised and delivered with high fidelity? Would my young person have had the same access to the treatment, the same family support, and the same freedom from competing demands?

### 8.8 Transparency

Are the search strategy, the inclusion criteria, the extraction forms, and the analytic decisions reported in sufficient detail that the review could be replicated? A review in a high-impact journal with no search strategy and no protocol is a narrative review with a pooled number attached.

## 9. The Application of Evidence to an Individual Case

A student who has learned the framework above will still face the situation in which the evidence for the young person in front of them is thin, contested, or absent. The following principles govern that situation.

**Treat the evidence as a prior, not a verdict.** The literature establishes base rates and reasonable approaches; it does not establish what will work for this young person. The clinician's task is to combine the evidence with the case, and the combination must be stated explicitly in the formulation so that it can be reviewed.

**Match the strength of the claim to the strength of the evidence.** If the evidence is weak, the plan should be correspondingly conservative, more measurable, and more frequently reviewed. Weak evidence justifies more frequent monitoring, not more confident assertion.

**Measure and review.** A treatment that is being used in the absence of strong evidence for the particular young person should be monitored systematically from the outset, with a pre-specified review point, and the young person should be told that this is what is happening and why. This is honest, and it is more respectful of the young person's autonomy than a confident assertion unsupported by any evidence.

**Prefer the reversible.** Where several approaches have comparable or absent evidence, prefer the approach whose effects can be observed and adjusted within a short period.

**Be explicit about the uncertainty in the plan.** "We are going to try X because it is the best-supported approach for this kind of difficulty, we will know in four weeks whether it is helping, and if it is not we will change something" is a more professional statement than a confident claim that X will work.

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## Further Reading for This Chapter

- Society for the Psychological Study of Social Issues (2011). *Consensus statement on the evidence base for the effectiveness of psychotherapy*. — The most carefully worded consensus statement available, and a model of how to state a level of confidence accurately.
- Wampold, B. E., & Imel, Z. E. (2015). *How psychotherapy works*. Wiley. — The most comprehensive current statement of the research, with an unusually good treatment of practice-based research.
- Kiresuk, H., & Schwebel, A. I. (2018). *Creating a research agenda for the evidence-based practices*. — Useful for the implementation and policy material.
- Onu, C., & Chibnall, D. (2019). *Evidence-based practice: Core concepts and applications*. — A clear account of the framework that the codes intend.
- Society for Research on Child Psychotherapy (2021). Research and practice with children and adolescents. — The best available summary of the state of the child and adolescent research.

## 10. Reading a Meta-Analysis: A Worked Procedure

The procedure below is used in the seminar. It takes about forty minutes with a single review and produces a written appraisal of about 800 words, which is the format the annotated bibliography assignment requires.

### Step 1: Read the abstract, the conclusions, and the tables (10 minutes)

Read the abstract and the conclusions first, and then the characteristics table and the risk-of-bias figures. If the paper's headline claim does not survive the reading of the tables, that is the finding.

### Step 2: Find the question (3 minutes)

Write the PICO question in your own words: Population, Intervention, Comparison, Outcome. If the paper does not state a question that can be rendered in this form, note that the review is a narrative review and that the pooled estimate is decorative.

### Step 3: Trace the flow of studies (5 minutes)

How many records were identified, how many screened, how many included, and how many contributed data to each analysis? A large discrepancy between the number of studies included and the number contributing to the pooled estimate usually indicates that the pooled estimate rests on a small subset, and you should find out which.

### Step 4: Assess the risk of bias (8 minutes)

For each of the two or three included studies you examine closely, note the randomisation procedure, whether the allocation was concealed, whether assessors were blind, the attrition rate and its direction, and whether the protocol was delivered with fidelity. Then ask the question that matters: are these flaws likely to inflate the effect? High attrition with a differential rate between conditions inflates. Unblinded outcome assessment inflates. Poor fidelity dilutes, and dilution is a different problem with a different implication.

### Step 5: Examine the heterogeneity (5 minutes)

Find the $I^2$ statistic and the forest plot. If $I^2$ is high, identify the moderators that the authors propose and check whether they are plausible. A very high $I^2$ with no exploration of the sources is a serious defect, because it means the pooled number has no interpretation.

### Step 6: Assess the effect and its clinical importance (5 minutes)

Report the pooled effect with its confidence interval, in the units the authors used. Then translate: what does this mean for a young person with the severity described in the sample? Compare it with the natural course of the problem and with the consequences of no treatment. An effect that is small relative to no treatment may still be the best available, and that judgement requires the comparison the authors will often have made.

### Step 7: Consider the publication bias diagnostics (4 minutes)

Look at the funnel plot and at what the authors say about it. Note whether the small-study literature in this field is dominated by positive results — it is, in most areas, and the authors of a good review will say so.

### Step 8: Apply the ecological validity test (5 minutes)

Write down the answer to the question: *would this evidence apply to the young person in front of me?* List the similarities and the differences. This is the step that determines whether the review will change your practice, and it is the step that the profession's own literature most often omits.

### Step 9: Write the appraisal (5 minutes)

A useful structure is four short paragraphs: the question and the search; the quality of the included studies; the size and meaning of the effect; and the applicability to your practice, including a specific recommendation.

## 11. Case Illustration: A Treatment That Is Not Working, and How to Know

**The situation.** A student counsellor in a university placement is working with Jordan, a sixteen-year-old referred for "low mood and school refusal." Jordan's sessions are going well in the sense that Jordan attends, talks, and has a good relationship with the student counsellor. The student has been delivering a structured behavioural activation and problem-solving approach, adapted from a well-known manual, for six sessions. Jordan reports feeling "a bit better" on the weekly rating form, but has not returned to school and describes the sessions as "not really helping."

**The measurement.** The placement requires a Session Rating Scale and a standardised depression measure at intake, at session four, and at discharge. The intake score was in the moderate range; the session-four score is in the mild range; the young person's own ratings have moved from 2 to 5 on a ten-point scale.

**The supervision conversation.** The supervisor asks three questions.

*What did you expect, and by when?* The student answers: "I thought the mood would lift and he'd start going back in week two or three." The supervisor asks what the manual predicts, and the student finds the relevant section: the manual specifies that a substantial increase in activity is expected by session four and that the majority of participants show a reduction in depressive symptoms by session six, with some delay in the

return of school attendance.

*What is the functional relationship between the school refusal and the mood?* The student has not examined it. The supervisor asks whether the school refusal began before or after the low mood, and whether there is anything about the school, the timetable, a particular class, a particular teacher, or the journey that precedes the avoidance. The student reviews the notes and finds that the refusal began after a fall in mathematics and after a comment by a teacher that the student had described as "humiliating" — a fact recorded in the intake notes in the first session and not returned to.

*What do you know about Jordan's life outside the sessions?* The student answers that Jordan lives with his mother, who works shifts, and that there is a younger sister. The supervisor asks about the father's involvement, and the student discovers that this was not asked. The notes record only that the "parent" attended the first session.

**What the supervision produces.** Three conclusions.

First, on the evidence, the treatment is not yet disconfirmed. The self-report ratings have improved and the standardised measure has moved. The manual's own expected trajectory has not been exceeded, and the manual specifies a longer period before the school attendance change.

Second, the formulation is inadequate regardless of the manual. The manual was designed for a population whose depression was maintained by inactivity and whose functional deterioration followed it. Jordan's school refusal preceded the low mood and appears to be maintained by a specific avoided situation, and a behavioural activation approach that does not address the avoidance will produce exactly the pattern observed: improved mood, no change in school attendance. A manual applied to a case whose maintaining process is different from the manual's is a different intervention, whatever the fidelity.

Third, there is a missing piece of the assessment. The student's intake was incomplete on the family and developmental history, and the omission has had a specific consequence: the case has been formulated as a case of a withdrawn adolescent with depression rather than as a case of a boy whose school refusal is maintained by a feared situation, in a family with an unexamined structure, and possibly in the context of a separation.

**The revised plan.** The student counsellor and the supervisor agree on three things: a specific functional analysis of the school refusal, using the framework in Chapter 24; an examination of the mathematics situation, in which the teacher is involved with consent, and which is the first step of an exposure hierarchy; and a full family and developmental history, obtained in the next session, with attention to the father's absence or presence and its effect on the family structure. The behavioural activation continues, because it is helping with the mood and there is no reason to stop a helpful intervention; but the student now understands that it is treating a secondary problem and that the primary problem is untouched.

**The ethical dimension.** The supervisor raises a question the student had not considered. If the treatment had been allowed to run to its manualised conclusion and then discharged with the note "improved mood, low mood resolved, attendance unchanged," the outcome would have been recorded as a successful episode and Jordan would have left the service with the same problem with which he arrived. This is the most common form of harm in a service that measures only what it is treating: the young person's actual presentation is not the measure, and the measure becomes the goal. The requirement to measure the target of the intervention, and to review against it, is an ethical requirement and not merely a methodological nicety.

## 12. Evidence and Research Support

**Well supported.** That specific, competently delivered treatments produce superior outcomes to no treatment for many conditions. That some interventions have larger effects than others for particular conditions, with the clearest examples in anxiety, in conduct problems, and in eating disorders. That practitioner competence and the quality of the alliance account for a substantial proportion of outcome variance in routine services. That structured feedback improves outcomes. That the majority of the outcome variance in routine services is attributable to the provider rather than to the model.

**Supported with qualifications.** That the outcome of treatment is predictable from diagnosis; the prognostic power of diagnosis is modest, and the prognostic power of comorbidity, family functioning, and the severity of impairment is higher. That structured manualised treatment is superior to usual care; the superiority is small in most routine-service comparisons and depends on the fidelity of the delivery and on the quality of the comparator. That evidence-based practices are superior to usual care; the effect is real but modest, and the greatest gains come from the implementation supports rather than from the training alone.

**Weakly supported or contested.** That a specific theory is superior to another in general. That specific effects are negligible; the component literature does not support this. That the "specific effect" and "common effect" can be cleanly separated; the current methodological position is that they cannot be estimated independently in the same way, and that the alliance is a mediator rather than a competitor.

**Methodologically weak or absent.** Evidence for the treatment of specific populations of young people — including those with neurodevelopmental conditions, those who are out of care, those who are justice-involved, those who are experiencing poverty, and those who are young people of colour — with the partial and encouraging exception of the mindfulness, ACT, DBT-A, and multisystemic literatures. Evidence on the differential effectiveness of approaches for young people who differ by culture, by gender, or by neurotype. Evidence on the long-term outcomes of counseling. Evidence on the cost-effectiveness of counseling compared with other services, which is sparse and methodologically weak.

## 13. Strengths and Limitations of the Evidence Base

**Strengths.** The counseling research literature is, for a clinical field, large, methodologically sophisticated in places, and consistently transparent about its limitations. The therapy-outcome literature has produced genuinely important findings about alliance, about components, about the limits of the efficacy trials, and about the centrality of the practitioner. The practice-based research literature has done more than any other to keep the field honest about the conditions in which its work is actually done. And the field's measurement instruments, developed for routine clinical use, are among the field's most practically valuable contributions.

**Limitations.** The population problem is severe and systematic. The methodological quality of much of the literature is low by the standards of evidence-based medicine, and the field's own reviews consistently report high rates of uncontrolled designs and high attrition. Publication bias is substantial and is not adequately addressed by the standard diagnostics. The outcome measures are often poorly matched to what matters to young people. The treatment literature is overwhelmingly concerned with a small number of well-studied conditions, and the young people most likely to be referred to a school or community service are the ones least represented in it. And the field's public rhetoric about "evidence-based practice" is considerably more confident than its evidence warrants.

## 14. Cultural and Ethical Considerations

**Culture and the evidence base.** The evidence base is dominated by studies conducted in the United States, Western Europe, and Australia, with samples that are disproportionately white, disproportionately affluent, and disproportionately drawn from the intact nuclear family. The construct validity of the standard measures in a population with different assumptions about selfhood, family obligation, emotional expression, or the appropriateness of disclosing distress to an adult is a genuine methodological question, and the field's treatment of it is largely by assumption rather than by evidence. Chapter 8 develops this argument and Chapter 48 develops its ethical implications.

**The ethical requirement of measurement.** The codes of the profession require that practitioners work with "the most current and relevant evidence" and that they evaluate the effectiveness of their work. These two requirements together make outcome measurement an ethical obligation rather than an administrative one, and a practitioner who declines to measure on the grounds that measurement is bureaucratic is declining a professional requirement.

But the requirement carries a condition: the measurement must include the young person's own appraisal, and the measurement must be used in the service of the young person's work rather than in the service of the institution's audit. A young person whose only measured outcomes are produced by a clinician-rated scale, and whose own account of change is never collected, is being treated in a system that has decided not to listen to them.

**The ethical requirement of honesty about uncertainty.** The codes require that practitioners communicate with clients in a manner that is accurate and that does not misrepresent the effectiveness of the services provided. In a field where practitioners are under pressure to present confident rationales for their interventions, this requirement is more demanding than it appears. A practitioner who tells a fifteen-year-old "this approach has a very good track record with young people who are avoiding school" when the relevant evidence base is thin, indirect, and largely about a different population has misrepresented the effectiveness of the service.

**Equity as an evidentiary requirement.** A service that delivers the best-supported treatment to the young people who can access it most easily has not delivered evidence-based practice; it has delivered evidence-based practice to a selected population. Chapter 50 treats the distributive question, and Chapter 48 treats the power differential that determines it.

## 15. Key Terms

**Key Terms at a Glance**

- **Evidence-based practice** — The use, in clinical decisions, of the best available research evidence together with clinical expertise and the client's circumstances and preferences.
- **Empirically supported treatment** — A treatment that has met specified criteria for research adequacy, including replication in independent studies with comparison groups, defined populations, and manualised delivery; a claim about the state of the research rather than about superiority.
- **Efficacy** — The capacity of an intervention to produce better outcomes than a comparison condition under ideal, research conditions.
- **Effectiveness** — The capacity of an intervention to produce better outcomes than it would otherwise have in the routine conditions of ordinary services.
- **Implementation** — The process by which an intervention is adopted, delivered with fidelity, and sustained in a real service; the stage at which most of the loss of benefit occurs.
- **Efficacy–effectiveness gap** — The difference in average outcome between trials and routine services, attributable to case mix, practitioner competence, time pressure, comorbidity, engagement, and fidelity.
- **Pragmatic trial** — A randomised study conducted in routine care with the resources and constraints of routine care, which estimates effectiveness rather than efficacy.
- **Practice-based evidence** — Evidence generated by routine outcome monitoring in real services and analysed with quasi-experimental methods.
- **Learning health system** — A service in which routine data collection and comparison are used as the primary mechanism of quality improvement.
- **Randomised controlled trial** — A design in which allocation to conditions is randomised and outcomes are compared; the strongest available design for causal inference.
- **Control condition** — The comparison condition in a trial; its composition determines what the trial can estimate.
- **Credible control** — A control condition designed to reproduce the common elements of treatment while omitting the specific technique, so that the specific technique's contribution can be estimated.
- **Wait-list control** — A control condition providing no treatment elements; an inadequate basis for attributing outcome to a specific technique.
- **Attrition bias** — Systematic error arising when participants who leave a study differ from those who remain, typically in ways correlated with outcome.

**Key Terms at a Glance (continued)**

- **Selective reporting** — The publication of only those outcomes that reached significance, inflating apparent effects.
- **Publication bias** — The systematic under-publication of null or negative findings, producing inflated pooled estimates.
- **Funnel plot** — A diagnostic plot of effect size against sample size, used to detect publication bias; unreliable with few studies.
- **Heterogeneity** — The degree to which the results of studies within a meta-analysis differ beyond what chance would explain; measured by $I^2$.
- **Ecological validity** — The extent to which the conditions of a study resemble the conditions of the practice in which the findings would be applied.
- **Outcome measure** — The instrument used to assess change; the choice determines what is detected.
- **Session Rating Scale** — A four-item measure of session quality used both as an outcome measure and as routine feedback.
- **Natural course** — The trajectory a problem would have followed without treatment; the benchmark against which the clinical importance of an effect should be judged.

## 16. Chapter Summary

"Evidence-based practice" in counseling means combining the best available research evidence with clinical expertise and the client's circumstances, and the field's tendency to use the term to mean only "use a treatment that has been tested" is a source of confusion. The empirically supported treatment movement contributed a valuable methodological standard and a misleading label, and the implementation movement corrected the second problem by distinguishing the state of the research from the state of the practice.

The hierarchy of evidence organises the literature by internal validity, and the counseling literature occupies an unusual position within it, with its strengths in single-case designs, alliance research, and practice-based research, and its weaknesses in the population, the control condition, and the outcome measure.

The efficacy–effectiveness–implementation gap is the field's most important methodological insight. Trials show that treatments can work; routine-service research shows that they work less well, with large provider variation, under ordinary conditions. Practice-based evidence is the compass that tracks this.

For a profession that intends to work with children and adolescents, the population problem is the most serious: the adolescent evidence base is narrower than the adult one, the component literature is almost entirely adult, drop-out is high and non-random, and the field's habit of transferring adult findings without comment is an unacknowledged extrapolation.

A forty-minute appraisal framework, a worked case in which a helpful treatment is shown to be treating the wrong target, and the ethical requirement to measure the young person's own appraisal and to state uncertainty honestly together constitute the practical content of this chapter.

## 17. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish the two meanings of "evidence-based practice" and state which one the professional codes intend.
2. What did the empirically supported treatment criteria contribute, and what does the label not assert?
3. Explain the difference between efficacy and effectiveness, and name three factors that account for the difference.
4. Why did the field's early trials overstate the contribution of specific technique, and what design corrects this?
5. What are the advantages and the limitations of practice-based evidence? Why is it described as a compass rather than a map?
6. State four features of the counseling research literature that make it difficult to evaluate work with adolescents.
7. Why does the use of standard symptom scales systematically favour the common factors position?
8. Work through the eight-step appraisal procedure in Section 10 as it would apply to a review of mindfulness-based interventions for adolescent anxiety. What would you look for at each step?
9. In the Jordan case, what would have happened if the placement had measured only depression? What is the general form of that failure?
10. What does the profession's ethical requirement of accurate communication require of a practitioner in a case where the evidence is thin?

## 18. Practice Activities

**Practice and Application**

### Activity 7.1 — The Forty-Minute Appraisal (3 hours, individual, submitted)

Obtain a recent systematic review of a psychological or behavioural intervention for adolescents, using the library databases, and produce the 800-word appraisal described in Section 10. Submit it with the review's reference. The seminar will compare appraisals of the same review, and the differences in judgement between students are the most instructive part of the exercise.

### Activity 7.2 — The Evidence Map for a Presenting Problem (2 hours, groups)

Each group takes one common adolescent presenting problem — school refusal, panic, self-harm, family conflict, social withdrawal, substance use — and maps the evidence: what is well supported, what is supported with qualifications, what is contested, what is absent. Use the PICO format to structure the search, and report on the quality of what you find. The groups present in ten minutes, and the map becomes the group's reference for the formulation assignment later in the course.

### Activity 7.3 — The Unmeasurable Session (60 minutes, pairs, reflective)

Practise a session in which the young person reports a small improvement that the practitioner does not believe, and the practitioner must explore the discrepancy without dismissing the young person's account and without inflating it. The pair then reflects on what it feels like to hold both, and on the pressure to produce a satisfying session. The exercise is aimed at the difference between measurement and its corruption, which is the most common way that a service's data become useless.

## 19. Annotated Reading Pathways

**For the frameworks.** Read the evidence-based practice framework literature, and read the consensus statement on the evidence base for psychotherapy. Note the difference in tone between the two; the consensus statement is written by people who have to be accurate about a field they work in, and its careful hedging is a model for how to write about one's own evidence.

**For the pragmatic and practice-based literature.** Read the large pragmatic trials and the practice-based research reports. These are the studies that changed what the field believes, and they are also the studies with the most interesting internal disagreements.

**For the adolescent specifics.** Read the child and adolescent psychotherapy handbooks, which summarise the state of the sub-field, and then check the summaries against the primary studies. The summaries are considerably more confident than the studies support.

**For the critical stance.** Read the methodological critiques of the psychotherapy research literature — the papers on publication bias, on the exclusion of dropouts, on the equivalence-trial design, and on the overstatement of the evidence base in public policy. A student who has read only the favourable literature will over-trust it.

## Practice Laboratory and Extended Worked Example

This laboratory turns **The Evidence Base: Evaluating Counseling Theories** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is outcome research, treatment manuals, and the appraisal of efficacy for youth-serving interventions. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or

institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Nia is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Nia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Nia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Nia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Nia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Nia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for The Evidence Base: Evaluating Counseling Theories

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **The Evidence Base: Evaluating Counseling Theories** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **The Evidence Base: Evaluating Counseling Theories** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

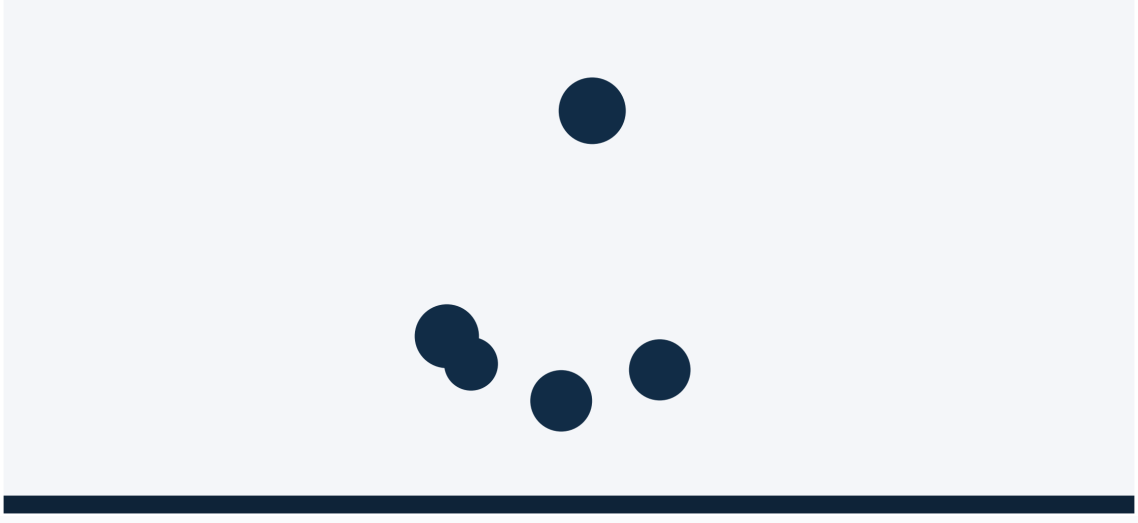
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **The Evidence Base: Evaluating Counseling Theories**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain The Evidence Base: Evaluating Counseling Theories to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 8

# Culturally Responsive Theory and the Cultural Lens

Culture as a constitutive element of personality, not a variable added to it.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the three generations of multicultural counseling theory and the assumption that separates them.
- Distinguish the concepts of culture, ethnicity, race, social identity, and acculturation, and use them accurately.
- Summarise the empirical findings on the relationship between cultural factors and mental health outcomes, and identify where the evidence is strong and where it is thin.
- Apply the cultural lens to a case, moving between the individual, interactional, family, community, and societal levels of analysis.
- Describe the structure and use of the cultural formulation interview, and explain its ethical function.
- Evaluate the evidence base for the three models of cultural identity development.
- Analyse how cultural factors modify the operation of a specific counseling theory, and identify which theories have been most and least developed for cultural responsiveness.
- Apply the principles of cultural humility to your own practice, including the assessment of your own assumptions.

## Chapter Overview

Almost everything in this book assumes a particular kind of young person. The assumption is rarely made explicit, and it is rarely examined: the young person is an individual; the family is a context rather than a client; the significant adults in the young person's life are the parents; the distress is located inside the person; the resources for change are the person's own; the culture is a variable that may modify the presentation of the problem but not the existence of it; and the professional's own values, categories, and authority are the appropriate frame for understanding all of it.

This chapter takes that assumption apart. It describes the development of multicultural counseling theory through three generations, sets out the vocabulary with which the field discusses culture, and reviews the evidence on culture and mental health, which is more substantial than is often acknowledged and less supportive of simple conclusions than is frequently claimed. It then develops the cultural lens as a practical instrument: a way of analysing a case at five levels, and of asking the question that a culturally responsive formulation must answer — *what is happening in this young person's life, and what would the difficulty look like if it were not a problem?*

The chapter also addresses the practitioner. Cultural responsiveness is not a set of techniques for working with young people from minority backgrounds; it is a self-examination of the framework in which the practitioner has been trained, and a set of disciplines for examining the categories, assumptions, and authority that the framework carries. A practitioner who can work skilfully with a young person from a different background and who has never examined the assumptions of their own training has learned a technique, not a discipline.

## 1. Three Generations of Multicultural Counseling Theory

## 1.1 The first generation: cultural sensitivity and cultural competence

The first generation, dominant from the 1970s through the 1980s, was built on two propositions. The first was that the mainstream theories and the mainstream practitioner were **anthropocentric** — that they took the norms, values, and worldviews of the dominant culture as universal, and treated deviation from them as pathology. The second was that the remedy was **cultural competence**: the practitioner should acquire knowledge of the beliefs, values, customs, and communication styles of the cultural groups they were likely to work with, and modify their practice accordingly.

The first generation made an important contribution. It named the anthropocentrism, and it was largely responsible for the curricular reforms that made culture a required element of counselor training. Its assumptions, however, are now generally regarded as inadequate in two respects.

The first is that **culture is treated as a static attribute of a group**. The "culture" of a manual or a workshop is presented as a set of facts about a population, homogeneous within itself and fixed over time. The research on cultural identity development, on biculturalism, on acculturation stress, and on intra-group and inter-group variation demonstrates that this is false, and the practitioner who has learned a cultural profile will encounter young people who do not fit it.

The second is that **the practitioner is positioned as the expert and the cultural group as the object of study**. A model in which the practitioner learns about a group in order to work more effectively with it reproduces the asymmetry of the clinical encounter, and it does so at the level of knowledge. It is not enough for the practitioner to know a great deal about a young person's background; the question is what the practitioner does with that knowledge, and a practitioner who has been given a great deal of information is at risk of using it to explain the young person rather than to understand them.

## 1.2 The second generation: cultural awareness and cultural humility

The second generation, emerging from the late 1980s, shifted the emphasis from the culture of the client to the culture of the practitioner. Its central concepts are **cultural awareness** — the recognition that the practitioner has been socialised into a particular cultural framework and carries assumptions that are not visible from within it — and **cultural humility**, which adds the requirement that the practitioner remain continuously open to the possibility that their framework is partial, and that the people they work with hold knowledge they have not been taught.

Cultural humility differs from cultural competence in an important respect: it is not a state to be achieved and then certified. It is a lifelong practice, and the second generation's own literature has become markedly more willing to say that no practitioner ever arrives at a state of cultural competence, and that the goal is rather a stance of informed humility in which the practitioner is always prepared to be corrected by the people they work with.

## 1.3 The third generation: cultural responsiveness and the social justice commitment

The third generation, which has become dominant in the twenty-first century, reframes the problem in structural terms. Its two central commitments are:

**Cultural responsiveness** — the proposition that no intervention is culturally neutral, that every theory, every technique, and every assessment instrument carries assumptions about the person and about what constitutes a problem, and that the practitioner must identify those assumptions in the specific case and determine whether they hold for this young person.

**Social justice and critical consciousness** — the proposition that individual distress is frequently produced by systemic conditions such as racism, poverty, colonisation, gender inequality, ableism, and the criminalisation of young people, and that a practice that locates all difficulty in the individual and all resources in the individual is a practice that will fail with the young people for whom the systemic conditions are the main cause.

The third generation also brings a distinctive ethical and epistemological stance, best described as **critical**: the practitioner asks not only whether the intervention is helping this young person but also whether the framework of the intervention reproduces the conditions that produce the young person's distress, and whether the young person's own account of their situation is itself constrained by a discourse that positions them as the problem.

Chapter 48 develops the third-generation position in full, and Chapter 49 applies it to a specific population. This chapter establishes the vocabulary and the evidence on which the later chapters depend.

## 2. The Vocabulary of Culture

The field's vocabulary has grown unwieldy, and much of the confusion in the professional literature comes from the imprecision of its terms. The following distinctions are the ones a student must be able to make.

**Culture** refers to the shared and transmitted patterns of meaning, values, norms, and practices that distinguish a group. Two cautions are necessary. The first is that culture is not a static possession of a person; people participate in multiple cultures — familial, regional, religious or secular, national, ethnic, racial, class, professional, gender, generational — and the configurations change over a life. The second is that the boundaries of a "culture" are themselves a matter of politics, and the classification of people into cultural groups has historically been a tool of administration and exclusion.

**Ethnicity** refers to shared heritage — a shared history, language, or ancestral origin — and is generally a matter of self-identification and of social ascription. The relevant clinical point is that ethnic identity may be central to a young person's identity, that it may be contested within the family, and that the level of identification is a developmental and contextual variable rather than a fixed attribute.

**Race** is a social construction with real material consequences. Treating race as a biological fact is a scientific error; treating it as socially meaningless is a political error that obscures the mechanisms by which race operates in health, in the justice system, in schooling, and in everyday interaction. The clinical consequence is that race operates as a predictor of the young person's exposure to stress, of the quality of the responses they receive from institutions, and of the probability that their distress will be misread. Chapter 48 develops this argument in detail.

**Social identity** refers to the intersection of multiple categories — race, gender, class, disability, sexual orientation, religion, and others — in the constitution of a person's position in the world. The concept of intersectionality, developed by Kimberlé Crenshaw in the context of the law and later applied to psychology, holds that the experiences produced by these categories cannot be understood by examining each separately, and that the structures of power that operate through them intersect in ways that produce

distinct experiences that are often invisible to the dominant account.

**Acculturation** refers to the process of change that follows contact between cultures, involving shifts in behaviour, identity, and social organisation. The widely used model of Berry describes four strategies — integration, separation, marginalisation, and assimilation — and the literature suggests that the integration strategy, in which the young person maintains both the heritage and the receiving culture, is associated with the best adjustment outcomes, while marginalisation is associated with the worst. The clinical relevance is that a young person of immigrant parents in a third generation may be moreaccurately described as having a bicultural identity than as being acculturating, and the therapeutic implication — that a bicultural young person may need help with the *negotiation* of two value systems rather than with the resolution of a conflict between them — is often missed.

**Microaggression, subtle discrimination, and cultural taxation** are the contemporary concepts that describe the cumulative effect of low-level discriminatory incidents on a member of a marginalised group, and the additional burden borne by members of such groups who are also responsible for representing their group to the dominant culture. These concepts matter clinically because the cumulative effect of repeated microaggressions on a young person's mental health is substantial and is frequently misattributed to individual psychopathology.

## 3. Culture and Mental Health: What the Evidence Shows

The evidence on culture and adolescent mental health is more substantial than the profession's ambivalence about it usually suggests, and it points in a consistent direction.

### 3.1 The relationship between racial and ethnic discrimination and mental health

The research on discrimination and mental health among adolescents is now extensive and consistent. Minority stress — the theory that the experience of chronic discrimination and of the threat of discrimination constitutes a chronic stressor with predictable mental health consequences — is supported by a substantial literature in adolescents, in adults, and across racial and ethnic groups, and the effects are detectable in both symptoms and in the objective markers of physiological stress. The relationship is dose-dependent, is stronger for discrimination that is chronic rather than episodic, and is stronger for discrimination that occurs in the domains in which the young person is most exposed to institutions — school, policing, healthcare.

Two findings are especially relevant to the practitioner. The first is that **the effect of discrimination on mental health is mediated substantially by the young person's appraisal of the discrimination as attributable to themselves**, and this is a point of direct theoretical interest: the mechanism is cognitive appraisal, which means the cognitive therapies may have something to offer here, provided they are applied to the appraisal of discrimination rather than to a self-blame that the discrimination has produced. The second is that **discrimination is associated with increased risk in the opposite direction to the one the field often assumes**: it is associated with higher rates of internalising symptoms, and the externalising and conduct problems that are more visible to the system are also elevated, which is a point that should temper any assumption that an adolescent's disruptive behaviour is not connected to their social position.

### 3.2 The protective role of ethnic and racial identity

The research on ethnic identity is consistent in a way that is important for practice: **a strong, positive, explored ethnic identity is associated with better mental health, better academic outcomes, and greater resilience**, and the effect is present across groups and across age. The protective effect is strongest where the young person's family and community reinforce the identity, where the young person has a sense of positive belonging, and where the wider society is not actively undermining it. Where the wider society is undermining it, the identity can be a source of strain as well as of strength, and the strain appears particularly at moments of racial incident or of public scrutiny.

The clinical implication is that the ethnic identity of a young person is not a marginal matter to be addressed at the end of an assessment but a central resource whose development may need active support, and that a practice that treats a young person's cultural identity as a source of risk to be managed has misunderstood the literature.

### 3.3 The role of the family and the community

The research on the protective role of the family, extended family, and community is strong across cultures, and it is the single most important finding for the practitioner working with an adolescent. The presence of at least one stable, reliable adult in the young person's life is one of the most consistent predictors of good outcomes across every problem domain and every population. For young people facing racism, poverty, displacement, or conflict, the presence of a connected and supportive adult is a stronger protective factor than almost anything else that a service can provide.

This finding has a direct theoretical implication for counseling, and it is the point on which the cultural critique of the individual framework is most concretely grounded: **the most effective intervention for many of the young people who present with the most severe difficulties is the repair of their social relationships**, not the alteration of their internal states. This is the point at which the cultural critique and the systemic tradition converge, and it is examined in Chapter 50.

### 3.4 Where the evidence is thin

The evidence is thin in several important places, and a student should know where. It is thin on the effectiveness of culturally adapted interventions as compared with cultural adaptations of the same intervention — the question of whether cultural adaptation improves outcomes is real but unresolved, and the field's claim that cultural adaptation is superior is asserted more often than demonstrated. It is thin on the evidence for particular cultural groups, because the research is organised around the categories of the dominant culture and the sample sizes for particular groups are frequently very small. It is thin on the relationship between cultural identity development and outcome in a way that is sufficiently controlled to guide practice. And it is thin on the long-term consequences of cultural identity, where the developmental and the political literatures have not been well integrated.

## 4. The Cultural Lens: Levels of Analysis

The cultural lens is a practical instrument for the formulation, and it works by requiring the practitioner to move explicitly between five levels of analysis. The formulation of a culturally responsive case should specify what is happening at each level and how the levels are connected.

[Figure placeholder — The cultural lens, showing the five levels of analysis in the cultural formulation of a case.]

**The individual level.** What is the young person's internal state? What are their appraisals, their emotions, their self-evaluations, their physiological state? The individual level is where the clinical tradition locates the problem, and the cultural critique does not deny that it matters; it denies that it is sufficient.

**The interactional level.** What happens between people in the young person's immediate environment? How does the young person communicate, defer, resist, negotiate? What are the norms of emotional expression, of directness, of eye contact, of silence, of disagreement, and of physical contact in the young person's family and community? A young person who is described as "non-compliant" in a school may be performing a norm of respect for adult authority that is entirely appropriate in the family, and the school has read it as defiance.

**The family level.** What is the structure of the family, and what are the family's own cultural resources? Family structure varies enormously across cultures — the extended family, the multi-generational household, the separated and reconstituted family, the kin-centred network — and a formulation that assumes a nuclear family with two parents will misdescribe a great many cases. Chapter 33 addresses the specific issues of working with parents and adolescents across cultural contexts.

**The community level.** What institutions surround the young person — school, faith or secular community, sports club, extended kin network, neighbourhood — and what is the quality of their connection to them? What is the young person's reputation, and how does it operate within the community? In some contexts the community is the young person's primary source of identity and support; in others, it is the source of pressure and of the demand for conformity that the young person is trying to escape.

**The societal level.** What is the young person's position with respect to structural conditions — the labour market, the housing market, the immigration status of the family, the criminal justice system, the welfare system, the education system, the presence of racism? This is the level at which the third-generation critique locates the primary causes of many presentations, and it is the level that the individual clinical tradition most consistently omits.

## 4.1 The question that the lens generates

The cultural lens produces one question that no individual level analysis can produce:

> ***If this young person's difficulty were entirely a product of their culture, their family, their community, and their position in society — with no individual pathology at all — what would we see?***

The purpose of the question is diagnostic. If the answer is "we would see exactly what we see," then the difficulty is not primarily a product of the social context, and the intervention should be directed primarily at the individual level. If the answer is "we would see something quite different," then the social context is a substantial part of the picture, and an intervention directed only at the young person's internal states will fail.

This is a hard question, and it is deliberately discomposing, because the answer is often that the young person's symptoms are entirely explicable as responses to conditions that no one in the room is going to change. The correct professional response in that case is not to abandon the young person and not to explain away the difficulty, but to work on the individual level as well as on the social level, and to be honest with

the young person about what is and is not available.

## 5. The Cultural Formulation Interview

The cultural formulation interview is a structured set of questions, developed in the DSM tradition and now widely used in clinical and research settings, whose purpose is to elicit the narrative and the cultural meaning of the presenting problem from the perspective of the young person and, with permission, of the family. It is reproduced in full in Appendix A.

Its structure is worth understanding, because the structure embodies a set of commitments.

**The identity question** asks the young person to describe themselves, with the explicit framing that the question is interested in the identities that matter to them and in the ones that do not. The ordering matters: the question comes first, before the problem, so that the practitioner hears who the young person is before hearing what is wrong with them.

**The explanatory models question** asks what the young person and their family think is causing the problem, what they think has helped in the past, and what they expect from treatment. This question is the most clinically useful in the whole set, and it is the one most often omitted. The practitioner who does not know what the family believes is about the cause of the difficulty does not know whether their explanation is shared, and an explanation that is not shared is an intervention applied to a person who does not believe in it.

**The psychosocial stressors and supports question** covers the domains of the cultural formulation: the young person's key social relationships, the disruptions, the transitions, and the losses; the social aspects of the young person's functioning; the cultural elements of the young person's environment; and the differences between the young person and the significant others in their views of the problem.

**The cultural elements of the clinician–patient relationship** asks how the young person expects to be treated, whether there are differences in gender, age, or other factors between them and the practitioner that matter, whether there are aspects of the relationship that the practitioner should change, and whether there are any other aspects of the relationship that the young person wishes to have addressed.

**The overall assessment** asks the practitioner to summarise the cultural and social formulation, the cultural and social stressors and supports, and the cultural aspects of the clinician–patient relationship.

The function of the interview is threefold, and all three are relevant to the ethical requirements of the profession. It is a **clinical instrument**, in that the explanatory models and the family views frequently change the treatment plan. It is a **quality instrument**, in that it surfaces mismatches between the practitioner's assumptions and the young person's, which is the most common source of treatment failure with young people from minority backgrounds. And it is an **ethical instrument**, in that it establishes the young person's own account as a legitimate and necessary input to the formulation, and it does so in a form that can be documented, reviewed in supervision, and audited.

## 6. Models of Cultural Identity Development

Three models of ethnic identity development are standard in the literature, and the course requires students to be able to describe and compare them.

**Phinney's multistage model** describes ethnic identity development as a progression through a **pre-identity** stage, in which the young person has not thought much about ethnicity and may have internalised negative stereotypes; an **identity exploration** stage, in which the young person is actively questioning and learning about their own and others' ethnic identities; and an **identity achievement** stage, in which the young person has a secure, positive, and well-integrated ethnic identity. The model is empirically well supported, and the finding that the stage is associated with outcomes — with better psychological functioning, better academic engagement, and better family relationships — is one of the more reliable findings in the multicultural literature. Its clinical use is to predict that a young person in the pre-identity stage is unlikely to respond to an intervention that requires them to work explicitly with cultural meaning, and that the exploratory stage is the appropriate time to support that work.

**Helms' model** describes the movement from **unexamined identity** through **identity crisis** to **identity integration**, and emphasises the different developmental tasks and coping mechanisms characteristic of each stage. Its particular contribution is to the development of an explicit model for the multicultural person, and to the observation that the stages of majority and minority group identity development are parallel but not identical.

**DeChick's model** adapts Phinney's model to the specific case of the biracial and bicultural individual, and is the model to consult for a young person with a mixed heritage who is navigating multiple communities.

All three models are heuristic, all were developed on limited samples, and all are used in the field somewhat more confidently than the evidence warrants. Their value to a student is that they provide a developmental language for a clinical observation, and that they make it possible to distinguish a young person's developmental task from a pathology.

## 7. The Limits of the Multicultural Framework

Three limits should be named.

**The limits of the "culture" as a unit.** The multicultural framework is organised around culture as a category of population, and the categories available are those produced by the census, the immigration system, and the academic interest of the dominant culture. A young person whose identity is organised around a sexuality, a disability, a class position, a subculture, a fandom, a faith practice, or a political commitment is not well served by a framework whose central variable is ethnicity. Chapter 49 treats this directly.

**The limits of the "competence" framing.** Even in its third generation, the framework retains the structure of the practitioner being the competent one and the cultural group being the object of understanding. The critical and postcolonial literature in counselling, psychology, and education has been clear that this structure reproduces the epistemology of the colonial encounter, and that a genuinely responsive practice requires the practitioner to hold their own framework as one framework among several, and to be genuinely willing to be organised by the young person's account of the situation rather than by the professional's categories.

**The limits of the evidence.** As Section 3 indicated, the research base for culturally adapted interventions is limited, the research base for particular cultural groups is thin, and the field's confidence outruns its evidence. The honest position is that cultural responsiveness is ethically required, theoretically indicated, and empirically under-supported — and that the ethical requirement stands even where the evidence does

not, because the alternative is to deliver an intervention whose assumptions have not been checked against the young person they will be delivered to.

## Further Reading for This Chapter

- American Psychiatric Association. (2017). *Diagnostic and statistical manual of mental disorders* (5th ed.). — The cultural formulation interview, with its accompanying narrative, is in the appendix on cultural formulation.
- Ponterotto, J. G. (2013). *Multicultural counseling theories*. In J. C. Cassell (Ed.), *Theory, research, and practice in multicultural counseling*. — A careful account of the theoretical and empirical position, with an unusually balanced treatment of the evidence.
- Helms, J. E. (2009). The literature on ethnic identity development. In M. E. Nimier (Ed.), *The Cambridge handbook of multicultural counseling psychology*. — The standard source for the models of identity development.
- Parada, A., & Suárez-Orozco, C. (2017). *Borderline self-obsessions*. — Not required reading but recommended, for the account of the cultural determinants of borderline pathology in a young person with a migrant background.
- Warner, R. M. (1998). *A useful mental health handbook for African Americans*. — Older, and written for adults, but a valuable demonstration of the practical value of a culturally specific knowledge base.

## 8. How Culture Modifies Each Major Theory

Culture does not simply add a variable to a theory; it modifies the theory's premises. The following summary of the points at which culture enters each tradition is offered as a practical check.

### 8.1 Cognitive theory

**The premise at risk.** Cognitive theory assumes that the person interprets experience and acts on the interpretation, and that a maladaptive appraisal is a misapplication of a rational capacity. The cultural difficulty is that the appraisal is formed within a social world that the practitioner does not share, and the "irrational" appraisal may be an entirely rational response to a world that is not as safe, predictable, or just as the practitioner assumes.

**Specific modifications.** The cognitive therapist must establish what the young person's appraisals are of *specific situations*, and must not import the practitioner's own situational appraisals. The distinction between a thought and a belief must be made in the young person's own conceptual system rather than in the therapist's. Behavioural experiments must be designed in the young person's actual world — a test of the prediction "people will treat me fairly" requires a situation in which the young person is actually likely to be treated unfairly if the belief is correct, and this may be uncomfortable to arrange. The cognitive work on discrimination must be directed at the *appraisal of the discrimination*, and the practitioner must be careful not to require the young person to appraise their own appraisal of their own racism, which is a demand that many young people experience as a demand to accept the discrimination.

**The most common error.** The cognitive therapist who treats a young person's distrust of authority as a cognitive distortion to be corrected. Sometimes it is. Often it is accurate.

## 8.2 Behavioural theory

**The premise at risk.** Behavioural theory assumes that behaviour is learned and maintained by contingencies, and that the contingencies are the appropriate target of intervention. The cultural difficulty is that the contingencies are structured by social arrangements — the distribution of authority, the discipline practices of the school, the policing of the neighbourhood — and that an intervention which modifies the young person's behaviour without modifying the structure will produce compliance without change, or change at a high cost.

**Specific modifications.** The functional analysis must include the institutional contingencies, and not only the family's. A young person's non-attendance may be maintained by a school response that the school has not recognised as a contingency. The token economy and the incentive systems described in Chapter 28 raise distinct ethical questions in a school with a racialised discipline policy, because the differential application of reinforcement reproduces the differential application of discipline. And the behavioural practitioner must be alert to the possibility that the "problem behaviour" is a form of resistance to a structure that the young person has every reason to resist.

## 8.3 Humanistic and existential theory

**The premise at risk.** Humanistic theory assumes the self-actualising tendency, and it assumes that the conditions of growth are the conditions of relationship. The cultural difficulty is that the humanistic framework's account of the person is a distinctly Western and individualistic one: it presupposes a bounded, self-directed, self-expressing individual whose authentic self can be discovered by looking inward. In many cultural contexts the self is constituted relationally, and the "authentic" self that the young person should be guided towards may be a self that is more thoroughly embedded in a network than the framework allows.

**Specific modifications.** The concept of congruence must be examined: whose definition of authenticity is in use? A young person who is deeply committed to a family's expectations and also privately distressed by them is not "incongruent" in any simple sense; the situation is a conflict between a relational self and a personal one, and the conflict is a developmental and cultural problem rather than a symptom. The humanistic practitioner's own cultural location must be examined, since the "unconditional positive regard" that the framework offers is a form of regard that the practitioner extends on the basis of a particular understanding of persons.

## 8.4 Systemic and family theory

**The premise at risk.** Systemic theory assumes that the family is the primary system, and that the practitioner can enter it and work with its patterns. The cultural difficulty is that "family" is not one thing, that the boundaries of a family are culturally variable, and that the practitioner who imposes a nuclear-family model on a system that is structured differently will misdescribe it.

**Specific modifications.** The genogram and the structural map must be drawn around the family's own actual structure, which may include a grandparent who functions as a parent, an aunt who functions as a parent, a family business, a religious community, or a transnational set of relationships. The practice of including the parents in the work with a young person, which the evidence supports in many circumstances,

must be examined for its fit: in a context where the family is a source of safety, excluding the family is a serious error; in a context where the family is a source of danger, including it may be dangerous. The genogram should also record the family's own history of migration, displacement, and loss, which is frequently the actual location of the difficulty.

### 8.5 Constructivist and narrative theory

**The premise at risk.** Constructivist theory assumes that the meaning a person makes is the appropriate object of work, and that the practitioner and the young person can negotiate a shared account. The cultural difficulty is that the practitioner holds institutional and professional power within the negotiation, and that the young person's account may be constrained by a discourse that positions them as deficient — a discourse that the practitioner, by virtue of professional training, is also inside.

**Specific modifications.** The narrative practitioner must be alert to the fact that the "dominant story" may be the dominant story of the practitioner's own culture, and that the young person's account is sometimes the only account that has not been suppressed. The practice of externalisation — separating the young person from the problem — is culturally sensitive, because the discourse that the practitioner externalises is the discourse that the young person is inside, and the young person may experience the externalisation as a subtle form of accusation.

## 9. Culture and the Therapeutic Relationship

The alliance literature reviewed in Chapter 5 is largely conducted with Western samples and largely operationalised in Western terms. The cross-cultural literature makes several adjustments that are directly relevant.

**The dyadic assumption.** The "therapeutic alliance" presupposes a two-person relationship. In many cultural contexts the significant relationship is not dyadic but triadic or collective, and the young person may expect that the family or the community is a party to the work. A practitioner who insists on a strictly dyadic frame may be excluding the very relationships that sustain the young person.

**The confidentiality expectation.** Confidentiality is one of the clearest cultural variants in the alliance literature. Many young people from collectivist or community-oriented backgrounds expect that what they say will be shared with the family, and may regard a strict confidentiality norm as evidence that the practitioner does not understand the family. A practitioner who imposes strict confidentiality without discussion may inadvertently communicate distrust of the family. The solution is to make the confidentiality arrangement explicit and to discuss it, which is the practice the cultural formulation interview supports.

**The idealisation of the practitioner.** In cultures with high respect for authority and for expertise, the young person may present an idealised and uncritical view of the practitioner, and the alliance may be strong precisely because the young person is not treating the practitioner as a person they can be honest with. The relationship is then not a working alliance but an idealisation, and the working material has to be found in a different way. This is a configuration that the psychodynamic and relational literatures describe and that the cross-cultural literature confirms.

**Verbosity and disclosure norms.** The norms of self-disclosure differ across cultures and across families, and a young person whose family values restraint will produce a sparse narrative that will be misread as a lack of engagement or a limited capacity for insight.

## 10. Culture and Assessment

Assessment with a young person from a minority background is where cultural failure is most likely and most consequential, and four specific problems recur.

**Diagnostic misreading.** The standard diagnostic criteria contain cultural assumptions about the phenomenology of conditions, and the effect of the cultural misreading is to inflate the diagnostic rate in minority populations, with consequences that follow from the diagnosis for treatment, for medication, for institutional referral, and for the young person's own self-understanding. The studies of psychosis in African and Caribbean populations, and of ADHD across cultural groups, are the best-documented examples.

**The family as the problem.** Where the family's cultural practice is unfamiliar to the practitioner — a practice involving a parent with a physical disability, a family that has recently migrated, a grandmother who is the primary carer — the difficulty can be misread as a family dysfunction. Chapter 33 addresses the specific dynamics.

**The neglect of the social determinants.** Standard assessments ask about the young person's internal state, their symptoms, their history, and their functioning, and they do not systematically ask about the conditions in which the young person lives. A young person who is not sleeping because the family has one room for four people and no heating, who is not attending because the journey is unsafe after a fight at school last year, and who is not eating because the family's income halved when the father lost his work, has a presenting picture that will be misread if the assessment does not ask.

**The instrument problem.** The standard measures were normed on Western, white, and often adult samples, and their use with a young person from a different background can produce a misleading score. This does not mean the instruments should not be used; it means that the score should be treated as one source of information, with the cultural meaning of the item examined before the number is interpreted.

## 11. Culture, the School, and the Family

Two institutions dominate the lives of young people and both are culturally positioned in ways that a culturally responsive practitioner must understand.

**The school.** The school is the institution with which a young person from a minority background is most likely to have a conflict, and the reasons are structural: the differential application of discipline, the differential treatment of the same behaviour by different teachers, the under-representation of minority staff, the linguistic mismatch between the young person's home language and the language of instruction, and the tendency to interpret cultural difference as behavioural deficit. A school referral, in other words, is itself a culturally meaningful event, and the practitioner should ask what has happened at school before interpreting the referral as evidence of the young person's disturbance.

**The family.** The family is the young person's primary system and, for many young people from minority backgrounds, the primary source of both stress and resilience. The cultural-responsiveness literature is clear that the family should be engaged where it is safe to do so and where engagement is not required as a condition of the young person's care, and the discipline here is real: a practitioner who insists on parental involvement as a precondition of treatment will exclude precisely those young people for whom the parents are part of the problem.

The practice of a culturally responsive family assessment is set out in Appendix A and developed in Chapter 33.

## 12. Case Illustration: The Fifth Formatter

**The referral.** A fifteen-year-old boy, Idriss, is referred by his school for "non-compliance, low mood, and possible ADHD." The referral states that "he doesn't listen, doesn't do the work, has detention every week, doesn't want to be in school, and his mum says he's the same at home." The school has previously considered a behaviour plan and has discussed, at a parents' evening, whether a diagnosis of conduct disorder might be appropriate.

**The background, obtained with consent.** Idriss is a recent arrival, having come to the area eight months ago, having lived with his grandmother and two aunts in another city while his mother worked. His mother, Fatou, is now working two jobs, one of which involves a night shift, and is in the process of applying for housing; she has been on the waiting list for eleven months. Idriss speaks a second language at home and has been at this school for eight months. He describes the school as "fine" and gives no more detail. He has not been physically aggressive, but he has refused to comply with three separate staff on at least six occasions each, and has been in detention "loads of times." His grades are poor in two subjects and average in the rest.

**The cultural formulation interview.** The practitioner asks the identity question first. Idriss names his family, his country, his language, and his football team, and does not name a country as a central identity. The explanatory models question is the pivotal one, and Idriss's answer is: "My mum says I don't try because I've decided I don't want to be here. I think she says that because she's tired. I think it's because the teachers talk like I can't understand them and I do understand them. I think they don't know I came from another school where you could ask questions."

**The formulation.** The practitioner now has a competing account, and the important work is to hold both without discarding the school's. The school's account — that Idriss is non-compliant and possibly has a conduct disorder — is not an unreasonable reading of a referral of this kind, and it is what a service that does not ask the explanatory models question would write. But the information now available supports a different formulation, and it is one that the referral did not contain.

The formulation is that Idriss's non-compliance is **a response to an experience of being treated as incapable of understanding**, in a school where his accent, his questions, and his previous educational history mark him as a foreign body, and where the school has responded to his non-compliance with detention, which he experiences as confirmation. The attendance data are consistent with this: Idriss's absences cluster in the two subjects in which he is weakest, and in those subjects the teachers are those who report that he "doesn't try." The low mood is consistent as well, though the practitioner notes that the mood data are derived from a rating scale that has not been validated in his second language, and treats them with corresponding caution.

The ADHD question is left open. The practitioner is explicit that there is no information supporting it — no information on early development, no information on the pattern across settings, no information from the previous school — and that the school has generated the referral partly because a diagnostic label would make the school unaccountable for a young person it does not know how to teach.

**What the practitioner does.** Three things, in this order.

First, she asks Idriss's mother to come to the next session, and asks Idriss whether that is all right, and does not tell him what she wants to discuss. She is careful, because she knows that the mother's account will be

challenged and that Idriss will know it. She also says to the mother, before the session, that her task is to understand what is hard for her and what she is managing, and that the school's account of the problem as a difficulty with Idriss is one of several, and that she would like to hear hers.

Second, she contacts the two subject teachers separately, with consent, and asks a functional question: what specifically does Idriss do when he does not comply, and what happens immediately before and after? She also asks them what happens with a student who asks a question. The answer is instructive, and she does not share it with the school's senior leadership, because the practitioner is not the school's agent and the teachers are not her clients.

Third, she does not make a diagnosis, and she does not accept the school's framing of a possible conduct disorder. She writes to the school, with consent, stating what she is doing, what she is not able to offer, and when she will report back. The letter is careful, non-confrontational, and specific.

**What happens.** Fatou's account is that Idriss is exhausted, that the school is fifteen miles from home and that the bus takes an hour, that she is working nights and that he is effectively a young person living alone, and that he was thriving at his previous school. The assessment of the extended family reveals that his grandmother, who is 71 and has diabetes, has been the person getting him up and fed, and that his mother has not been able to be home on weekday mornings since the second job started. The functional data from the teachers show that Idriss's non-compliance occurs almost exclusively in two classes, and that both teachers' responses to his questions are curtailed.

The plan that follows is not a treatment plan in the first instance. It is a set of practical arrangements — the bus, the timetable, the grandmother's capacity, the mother's shift pattern, the housing application — and the counselling work begins with the relationship and with Idriss's account of his own competence, which is the thing the school has been inadvertently attacking for eight months.

**The teaching point.** Nothing in the case required a new technique. It required the explanatory models question. A culturally unresponsive service would have received the school's referral, applied a deficit model, considered a diagnosis, and recommended a behaviour plan, and would have found the young man more difficult in a year. The difference between the two outcomes is a difference of formulation, and the formulation was available at the first session in a question that costs nothing to ask.

## 13. Evidence and Research Support

**Well supported.** That discrimination and minority stress are associated with adverse mental health outcomes in adolescents, with dose-dependent effects and a substantial mediating role for appraisal. That a strong, positive ethnic identity is associated with better mental health, academic engagement, and resilience. That the presence of a stable and supportive adult is one of the strongest protective factors across problems and populations. That cultural identity development follows a developmental sequence and is associated with a range of outcomes. That family and community connectedness is protective across cultures.

**Supported with qualifications.** That culturally adapted interventions produce better outcomes than their unadapted originals; the direction of the finding is generally positive and the effect is usually modest, and the studies are few and generally not of high quality. That the practitioner–client cultural match improves outcomes; the finding is inconsistent, and the stronger version of the claim — that match is a condition of effective work — is not supported by the evidence.

**Weakly supported or contested.** That bicultural stress is a distinct, measurable phenomenon with distinctive sequelae; the concept is theoretically useful and empirically under-specified. That acculturation gap is a cause of family conflict in immigrant families; the concept has been substantially revised in the light of criticisms of its assumptions about family structure and about the direction of cultural change, and the empirical findings are weak.

**Absent.** Robust evidence on the differential effectiveness of approaches for young people of different cultural groups; evidence on culturally adapted interventions with specific populations; evidence on the long-term effects of cultural identity development.

**A note on the strength of these findings.** The research on culture and mental health is, in general, of better quality than the research on psychotherapy effectiveness, and it is a considerable irony that a field which has spent three decades calling for evidence-based practice has produced a body of culturally sensitive research that is stronger than much of its own treatment research. A student should be able to say this without defensiveness: the evidence for the importance of culture is better than the evidence for the interventions the field delivers.

## 14. Strengths and Limitations of the Culturally Responsive Framework

**Strengths.** It identifies a genuine and consequential failure in the mainstream tradition: that theories and practitioners have taken the dominant group's norms as universal, and that this failure has produced measurable harm. It provides a set of practical instruments — the cultural formulation interview, the levels of analysis, the models of identity development, the minority stress framework — that can be used in real cases. It redirects attention to the social conditions of the young person's life, which is where the largest effects are. And it makes the practitioner's own assumptions an object of study, which is ethically necessary and practically useful.

**Limitations.** The evidence base for culturally adapted interventions is thinner than the claims. The concept of culture remains under-theorised in relation to the individual-level variables the clinical tradition measures, so the framework describes the social context well and the interaction between the social context and the individual poorly. The emphasis on the cultural "other" can, if not handled carefully, re-centre the dominant group's practices as the norm, which is precisely the problem the framework was developed to address. And the framework's emphasis on the practitioner's self-examination, while correct, can become a preoccupation that crowds out the practical work.

## 15. Cultural and Ethical Considerations

The ethical requirements of culturally responsive practice are specified in the codes of the profession, and the requirements are more specific than they first appear.

**Non-discrimination.** The codes prohibit discrimination on the basis of race, ethnicity, national origin, religion, sex, gender identity, sexual orientation, age, disability, and socioeconomic status. The prohibition extends to the assessment process, and a practitioner who applies a standard instrument without examining its cultural assumptions and without examining whether the resulting score is meaningful in the young person's context has discriminated in effect.

**Cultural humility and self-assessment.** The codes require the practitioner to examine their own assumptions, biases, and power. The requirement is ongoing and it is specific: the practitioner should be able to name, in writing, the populations they assume they understand well, the populations they assume they do not, and the kinds of young person whose case they are most likely to misformulate.

**Communication.** The codes require that the practitioner communicate in a manner that the client can understand, and that requirement has a cultural dimension that is easy to miss. Communication includes the interpretation of silence, of directness, of eye contact, of the appropriate register of address, of the use of humour, and of the appropriate degree of directness in discussing difficult matters. A practitioner who is comfortable with directness will misunderstand a young person whose cultural norm is indirectness, and will reach the wrong conclusion about the young person's insight, their engagement, or their honesty.

**Power.** The profession's codes increasingly require attention to the ways in which the counselor–client relationship is shaped by social power, and the third-generation literature insists that this is not optional. A practitioner working with a young person whose family has no secure housing, whose parents work two jobs, and who is at risk of school exclusion is working with a young person whose access to the treatment itself is a function of the family's economic position. A service that requires weekly attendance during a period of family crisis is requiring a resource the young person may not have. Chapter 48 treats this in detail.

## 16. Key Terms

### Key Terms at a Glance

- **Cultural responsiveness** — The requirement that an intervention be examined for the assumptions it carries about the person and about what constitutes a problem, and adapted where those assumptions do not hold for the young person.
- **Cultural humility** — A stance of continuous openness to the possibility that the practitioner's framework is partial and that the people they work with hold knowledge they have not been taught.
- **Cultural competence** — The first-generation construct in which the practitioner acquires knowledge of a cultural group in order to work more effectively with its members; now generally regarded as an inadequate model.
- **Anthropocentrism** — The assumption, identified in the first-generation multicultural literature, that mainstream theory and practice take the norms of the dominant culture as universal.
- **Minority stress** — The theory that the experience of chronic discrimination, and of the threat of discrimination, constitutes a chronic stressor with predictable mental health consequences.
- **Cultural taxation** — The additional burden borne by members of marginalised groups who are also responsible for representing their group to the dominant culture.
- **Microaggression** — A commonplace, low-level, often inadvertent instance of discriminatory intent or effect, which accumulates over time.
- **Ethnic identity** — A sense of belonging to and identification with an ethnic group, with an associated exploration of that group's history, values, and meaning; a protective factor when strong and positive.
- **Cultural identity development** — The developmental progression through which a young person examines, accepts, and integrates their cultural identity; modelled as pre-identity, exploration, and achievement.
- **Acculturation** — The process of cultural change following contact between cultures, involving shifts in behaviour, identity, and social organisation.
- **Biculturalism** — The state of holding and participating in two cultural systems, which is not a stage of acculturation but a configuration in its own right.
- **Acculturation gap** — The construct describing the divergence between the acculturation of parents and of children in an immigrant family; the empirical basis for the construct has been substantially revised.

**Key Terms at a Glance (continued)**

- **Intersectionality** — The principle that the systems of power operating through race, gender, class, disability, sexuality, and other categories intersect to produce experiences that cannot be understood by examining each category separately.
- **Cultural formulation interview** — A structured set of questions eliciting the young person's and family's explanatory models and the cultural meaning of the presenting problem; reproduced in Appendix A.
- **Explanatory models** — The young person's and family's own accounts of the cause of the problem, what has helped, and what is expected from treatment.
- **The cultural lens** — The five-level analytic frame — individual, interactional, family, community, societal — used to formulate a case culturally.
- **The social-context question** — The diagnostic question generated by the cultural lens: if the difficulty were entirely a product of the social context, what would we see?
- **Relational self** — A conception of the self as constituted through relationships rather than as a bounded individual, which is characteristic of many cultural contexts and is not accommodated by the individual-level clinical framework.

## 17. Chapter Summary

The mainstream counseling tradition has taken the norms, values, and categories of the dominant culture as universal, and the multicultural literature has named this anthropocentrism and developed three successive responses: cultural competence, cultural awareness and humility, and cultural responsiveness with a social justice commitment. The third generation is the contemporary position, and it makes two claims: that no intervention is culturally neutral, and that many presentations of adolescent distress are produced by systemic conditions rather than by individual pathology.

The vocabulary of the field — culture, ethnicity, race, social identity, acculturation, biculturalism, intersectionality — has been refined, and the evidence base on culture and adolescent mental health is stronger than the profession's ambivalence suggests: discrimination and minority stress are reliably associated with adverse outcomes, a strong and positive ethnic identity is reliably protective, and the presence of a stable and supportive adult is one of the strongest protective factors known. Where the evidence is thin — on the superiority of cultural adaptation, on differential effectiveness across cultural groups, and on long-term outcomes — the field's confidence outruns its data.

The cultural lens provides a practical instrument: a five-level analysis of the individual, the interaction, the family, the community, and the societal context, together with the social-context question that tests whether the young person's symptoms are explicable without individual pathology at all. The cultural formulation interview operationalises the approach and functions simultaneously as a clinical, a quality, and an ethical instrument.

The chapter's case, of a fifteen-year-old whose school referred him for non-compliance, was not resolved by a new technique. It was resolved by the explanatory models question, and by the practitioner's willingness to

let the young person's account compete with the referral's.

## 18. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the three generations of multicultural counseling theory and state the assumption that separates them.
2. Explain why the cultural competence model was considered inadequate, and what replaced it.
3. Distinguish ethnicity, race, and social identity, and give a clinical example in which the distinction changes the intervention.
4. Summarise the evidence on the relationship between discrimination and adolescent mental health, and explain the role of appraisal in the mechanism.
5. What does the research on ethnic identity as a protective factor imply for practice, and what does it imply for the design of services?
6. Work through the five levels of the cultural lens for a young person who is refusing to attend a school that is 45 minutes away by public transport, and whose parent works nights.
7. State the social-context question, and explain its diagnostic value.
8. List the components of the cultural formulation interview, and explain the ethical function of each.
9. Identify the point at which culture modifies the premise of the cognitive theory, and describe the specific error that most commonly results.
10. In the Idriss case, what was the single most important piece of information, and what would have happened without it?

## 19. Practice Activities

**Practice and Application**

### Activity 8.1 — The Cultural Formulation Interview Practicum (2 hours, triads)

One student plays a fourteen-year-old who has recently migrated and is failing at school; one plays the parent, who interprets the school as hostile; one conducts the interview using the questions in Appendix A. The interviewer is required to ask the explanatory models question and to ask the social-context question in the form given in Section 4.1. The observers produce a one-page formulation at all five levels, plus a list of three questions the interviewer should have asked. Rotate the roles. The parent role is the hardest and the most instructive.

### Activity 8.2 — The Assumption Audit in a Clinical Case (90 minutes, individual)

Take a case you have read in this book — Nadia, Emeka, Priya, Jordan, or Idriss — and write the five-level cultural formulation. Then write, in a separate column, the formulation you would have given from the standard clinical framework, with no cultural lens applied. Mark every place where the two differ, and specify what difference the cultural lens would make to the plan. The exercise is best done in writing because the differences are usually invisible in discussion.

### Activity 8.3 — The Practitioner's Own Map (2 hours, individual, confidential)

Draw your own five-level map: your family of origin, your cultural and linguistic background, the communities you belong to and do not belong to, the faith or secular commitments you hold, the professional training you received and the assumptions it installed, and the populations for which you predict you will misformulate. Then write the three assumptions you are most likely to impose on a young person. Keep this document; it is the raw material for the reflective journal in the course assessment and for the work in Chapter 48.

## 20. Annotated Reading Pathways

**For the theory and the history.** Read the multicultural counselling theory chapters in the standard survey texts, and then read the critical literature on the cultural competence model's limits. Note how quickly the field's language has changed in thirty years, and consider what each change was responding to.

**For the empirical base.** Read the primary studies on minority stress and adolescent mental health, the studies on ethnic identity and resilience, and the protective-factor literature on the presence of a supportive adult. The last of these is the most directly useful to a practitioner, and it is the least often cited.

**For the instruments.** Use the cultural formulation interview in practice, and read the manual that accompanies it. Then read the clinical literature on cultural formulation in specific populations, which is where the instrument's limitations appear.

**For the ethical work.** Read the professional codes' sections on non-discrimination and on cultural responsiveness, and read the third-generation and critical literature on power in the counseling relationship. This is the strand of the course in which the student's own conclusions are most likely to require revision, and it is best done with a peer group rather than alone.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Culturally Responsive Theory and the Cultural Lens** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is culture as a constitutive element of personality, not a variable added to it. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Marcus is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Marcus's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Marcus explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Marcus chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Marcus has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Marcus can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Culturally Responsive Theory and the Cultural Lens

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Culturally Responsive Theory and the Cultural Lens** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Culturally Responsive Theory and the Cultural Lens** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

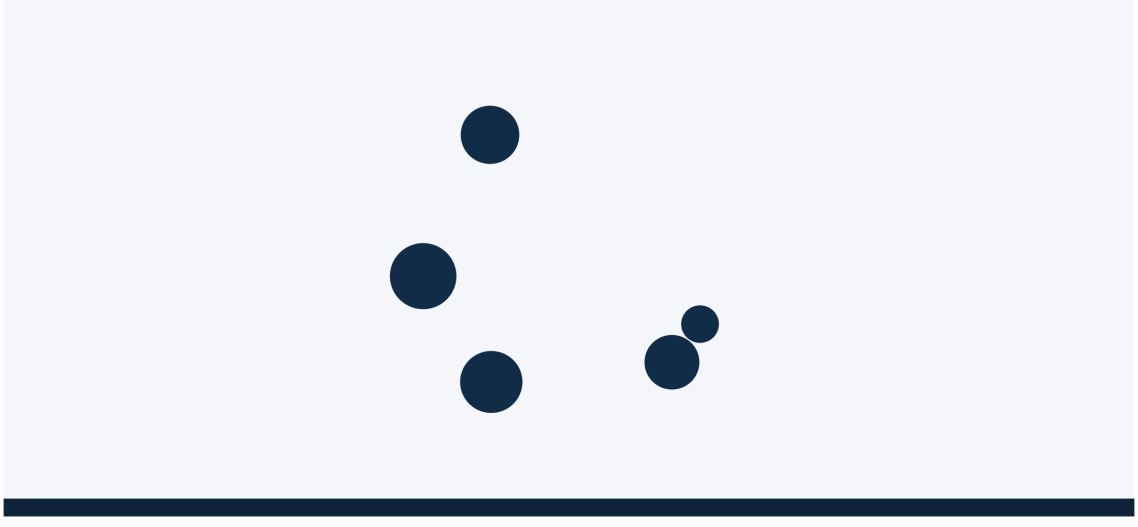
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Culturally Responsive Theory and the Cultural Lens**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Culturally Responsive Theory and the Cultural Lens to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 9

# Developmental Foundations for Counseling Youth

The developmental science that constrains and informs any theory applied to children and adolescents.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain why a theory of counseling developed with adults cannot be transferred to children and adolescents without developmental modification, and identify the modifications in question.
- Describe the major developmental frameworks relevant to practice with young people and the specific clinical contribution each makes.
- Explain the cognitive-developmental changes that occur across the second decade and their implications for the use of cognitive techniques.
- Describe the adolescent identity task, the psychosocial tasks of the school-age years, and the clinical significance of each.
- Explain the developmental course of attachment, the attachment classifications, and the clinical implications for work with adolescents.
- Summarise the neurodevelopmental evidence on the adolescent brain and distinguish the defensible claims from the popular ones.
- Apply the developmental perspective to a case, and identify the points at which a therapy has been misapplied to a developmental stage.
- Construct a developmentally appropriate formulation for a young person at a specified stage.

## Chapter Overview

Every theory in this book was developed with adult clients, and the majority were developed with adult clients in outpatient settings in the United States and Western Europe. The student who applies a cognitive therapy developed for a thirty-year-old's depressive appraisal to a nine-year-old's refusal to speak will produce a specific and predictable failure, and it is important to be able to specify that failure rather than merely to note that "developmentally inappropriate" is a phrase.

This chapter supplies the developmental knowledge required to make those modifications. It covers the frameworks that a counselor working with young people needs — the cognitive-developmental, the psychosocial, the attachment, the neurodevelopmental, the family, the moral, and the social — and it organises them around a single clinical question: *what is developmentally expected at this stage, and what is the meaning of the young person's deviation from it?*

The organising argument of the chapter is that the developmental perspective is not a set of facts to be recalled but a discipline of interpretation. A ten-year-old's concrete operational thinking is not a deficit against an adult standard; it is the expected form of thought at that stage, and what it means depends entirely on what the task demands. A fifteen-year-old's risk-taking is not a failure of parenting; it is a probabilistic decision made by an organism with an asymmetry between the reward and the penalty systems. A thirteen-year-old's belief that the world is fundamentally unfair may be a stage-appropriate response to evidence rather than a cognitive distortion. Developmental knowledge, used well, replaces moral judgement with interpretation, and it is the foundation on which the developmental adaptations in every subsequent part of this book rest.

## 1. Why Development Is a Condition of Theory

A developmental perspective has four distinct functions in counseling with young people, and they are frequently confused.

**Interpretation.** The developmental perspective provides the reference point against which behaviour is interpreted. A young person's behaviour is only interpretable against what is expected, and "expected" is a developmental judgement. Without it, practitioners default to moral and cultural judgements, and those judgements are usually the least informed and most biased judgements available.

**Selection.** Development determines which interventions are viable. A child cannot participate in a thought record that requires abstract propositional thought; a school-age child cannot reliably do interoceptive exposure; a young person cannot consent to a procedure whose rationale depends on a concept they do not have. The developmental perspective constrains the treatment plan directly.

**Dosage and timing.** Development determines the pace. The same technique delivered to a nine-year-old and a seventeen-year-old requires a different schedule, a different session length, a different degree of parental involvement, and a different horizon over which change is expected.

**Meaning.** Development determines what the young person is doing. Adolescence is a period in which enormous change is occurring, and much of what presents as pathology in a young person is a normal developmental process that has become stuck or has become pathological in its own right. The diagnostic question is never "is this abnormal?" but "is this developmentally appropriate, and if not, in what way?"

## 2. The History of Developmental Psychology in Counseling

The relationship between developmental psychology and counseling has a notable history, in which the two disciplines were repeatedly connected and repeatedly separated.

The child study movement of the late nineteenth and early twentieth centuries established the normative approach: the description of what typical children do at typical ages, and the clinical use of the deviation from the norm. Its limitations were substantial — small samples, unselected, and largely of the privileged — and they have never been fully resolved.

The developmental psychology of the 1930s and 1940s, in the form of the work of Jean Piaget, the normative and clinical work of Arnold Gesell, and the psychoanalytic contributions of Anna Freud and of Erik Erikson, supplied the conceptual framework for the child guidance clinics, and the child guidance clinic is the institutional ancestor of modern child practice.

The 1950s and 1960s brought the "developmental psychopathology" perspective, associated principally with the work of DSM-II and the first DSM-III, which located the origins of adult disorder in childhood and made the developmental course of a disorder a central concern of classification. This work was the most intellectually consequential in the field's history and it is also the work that has been most distorted by subsequent use: the reification of categories in the third edition of the diagnostic manual, and the resulting practice of diagnosing a child with a disorder of unknown course.

The 1980s and 1990s brought the developmental systems framework, associated with the clinical applications of Vygotsky's work and with the developmental psychopathology of the Rochester group, and the field's contemporary attention to the transactional model — in which the child is an active agent in her

own development and in which the direction of influence between the child and the environment is reciprocal.

The contemporary position is that developmental psychology and counseling theory must be integrated at the level of the case, and that the integration is a professional skill rather than a theoretical given.

# 3. The Cognitive-Developmental Framework

## 3.1 Piaget's stages and their clinical relevance

Piaget's account of cognitive development has four stages, and the clinical relevance of each is different.

**The sensorimotor stage (birth to approximately two years)** is of limited direct relevance to the counseling practitioner, though it is foundational for the attachment literature and for the understanding of early adversity.

**The preoperational stage (approximately two to seven years)** is characterised by symbolic thought, egocentrism, centration, and the absence of conservation and of reversible operations. The clinical significance is considerable: the child at this stage cannot hold two perspectives at once, cannot reason backwards from an outcome to a cause, cannot understand that the amount of liquid is unchanged when the shape of the container changes, and cannot reliably distinguish appearance from reality. A young child who is told that a clinician will not tell their mother what they say will not be able to reason about the future consequences of the disclosure, which is one reason why confidentiality with very young children must be explained in concrete, immediate terms rather than in principle.

**The concrete operational stage (approximately seven to eleven years)** brings logical operations on concrete material, the ability to conserve, the ability to take others' perspectives, and the beginning of decentration. The clinical significance is that the child at this stage can think logically about concrete situations and can be engaged in problem-solving, rehearsal, and behavioural experiments, but cannot reliably work with abstract propositions, hypothetical situations, or generalisations. This is the stage at which cognitive techniques first become usable, and they must be concrete, visual, and tied to a specific recent event.

**The formal operational stage (approximately eleven and onwards)** brings abstract reasoning, the ability to hypothesise, the ability to consider possibilities, and the capacity for metacognition. The clinical significance is that the young person can now examine evidence, generate alternatives, and evaluate their own thinking — but the capacity is new, uneven, and highly sensitive to context. The adolescent who can reason formally about a hypothetical in a classroom may be entirely unable to do so about their own panic, which is the case because the formal operations are applied to material the young person can control and not to material that is emotionally charged.

**The crucial clinical principle.** The stages are not a ladder to be climbed and not a rigid sequence. A young person's capacity is context-dependent, stress-dependent, and topic-dependent, and a practitioner who concludes "she is too young for cognitive work" from her age alone is as likely to be wrong as one who concludes "she is old enough" from the same data. The developmental question is not "what stage is she in?" but "what can she do with this particular material, today, in this state?"

## 3.2 Vygotsky and the zone of proximal development

Vygotsky's contributions to counseling are less familiar than Piaget's and considerably more useful. The **zone of proximal development** is the distance between what a child can do alone and what she can do with guidance from a more knowledgeable other. Its clinical significance is that it identifies the optimal difficulty of a task: a task within the zone can be completed with help, and the help is the mechanism of learning. A task below the zone is trivial and produces boredom; a task above the zone produces helplessness and withdrawal. This is the developmental justification for the graded task hierarchies in behavioural approaches, and it generalises far beyond them.

The related concept of **scaffolding** — the provision of temporary support that is progressively withdrawn as the young person becomes capable — describes the pedagogical relationship in a way that maps directly onto the counselling relationship in work with young people, and it is one of the few places where the developmental literature and the therapy research literature converge cleanly.

The third contribution is the **cultural-historical** account of development, in which higher mental functions originate in social interaction and become internalised. Its clinical significance is that a young person's "intrapsychic" state is partly a record of prior interactions, and that the developmental tasks of adolescence are themselves culturally organised. Chapter 48 draws out the implication.

### 3.3 Information processing and executive function

The third contribution of the developmental cognitive psychology is the account of executive function: working memory, inhibitory control, cognitive flexibility, and planning. The clinical relevance is substantial and often overlooked. A young person with a neurodevelopmental condition may have a working memory that cannot hold a thought record in mind while they examine it, an inhibitory control that cannot suppress an intrusive worry, or a flexibility that cannot generate an alternative appraisal. The cognitive therapist's failure to understand this will lead to a misformulation of what is a motivational problem as a belief problem, and to the delivery of a technique that the young person is developmentally unable to execute.

The clinical implication for technique design is specific: reduce the load on working memory, externalise the material, and structure the task so that each step has one demand on it.

## 4. Psychosocial Development: Erikson and the Contemporary Extensions

Erikson's eight stages remain the most widely used organising framework for psychosocial development in counseling practice, in part because they are memorable and in part because the field has not fully absorbed the more rigorous alternatives. Their clinical value lies in the specification of the developmental *task* and the *virtue* whose failure or distortion characterises the stage, which gives a practitioner a way of describing a young person's difficulty that is developmental rather than moral.

| Stage | Age | Task | Distortion when unresolved | Clinical presentation in counseling |
|---|---|---|---|---|
| Infancy | 0–1.5 | Trust versus mistrust | Mistrust | Difficulty trusting adults; guardedness; expectation of abandonment |

| Stage | Age | Task | Distortion when unresolved | Clinical presentation in counseling |
|---|---|---|---|---|
| Early childhood | 1.5–3 | Autonomy versus shame and doubt | Shame and doubt | Excessive shame; self-doubt; fear of asserting; toilet and autonomy struggles |
| Play age | 3–6 | Initiative versus guilt | Guilt | Guilt-driven perfectionism; difficulty with unstructured play; inhibition of assertion |
| School age | 6–12 | Industry versus inferiority | Inferiority | Academic and social inadequacy; competence doubts; withdrawal from challenge |
| Adolescence | 12–18 | Identity versus role confusion | Role confusion | Diffusion; instability; inability to commit; identity foreclosure |
| Young adulthood | 18–40 | Intimacy versus isolation | Isolation | Disconnection; difficulty with committed relationships; shallow connection |
| Adulthood | 40–65 | Generativity versus stagnation | Stagnation | Meaninglessness; unproductive contribution; mid-life disengagement |
| Later adulthood | 65+ | Integrity versus despair | Despair | Regret; bitterness; fear of death; loss of meaning |

Erikson's framework is used in this book as a heuristic and not as a diagnostic instrument, and the reasons are worth stating: the stages are not empirically derived, the age boundaries are culturally variable, the resolution of a stage is not a binary event but a lifelong process, and the framework was developed in a particular cultural and historical context. The contemporary developments that have extended it are more useful.

**The adolescent identity task** has been substantially revised since Erikson. Current understanding holds that identity in adolescence is not a single achievement but a set of ongoing commitments, explored in multiple domains, that are revised in the light of experience and are not expected to be permanent. The clinical significance is that a young person who is uncertain about their vocational direction, their sexual orientation, their peer group, or their political commitments is not pathological and does not require intervention. What requires intervention is *foreclosure* — a premature and rigid commitment, often made under pressure from family or community, which forecloses further exploration. The clinical task is often to make the possibility of exploration available rather than to resolve the uncertainty.

**The increase in autonomy with age.** The developmental literature on the second decade describes a consistent increase in the young person's autonomy, in the sense of their capacity to make and carry out decisions, and a concurrent increase in conflict with parents. The widely replicated finding of a "mismatch" between the adolescent's and the parent's eveningness chronotype is a small and charming illustration of a general truth: the adolescent is developmentally out of step with the adults around them, and this is not a problem to be solved but a phase to be negotiated. Parent–adolescent conflict that is low in intensity and high in frequency is normal, and a practitioner who treats all conflict as pathological will either pathologise ordinary development or fail to detect the conflict that is genuinely dangerous.

**The self and the "possible self."** The developmental research on the possible self — the version of oneself one is trying to become, along with the fear of becoming it — has direct clinical relevance. Young people's anxiety and depression are strongly associated with the discrepancy between their present and possible selves, and the therapeutic work that follows from this is a values-and-identity work rather than a symptom work. Chapter 37 on emotion-focused therapy and Chapter 51 on integration both draw on it.

# 5. Attachment and Its Developmental Course

## 5.1 The theory

The attachment theory originated in John Bowlby's work on the effects of separation and on the motivational systems underlying proximity-seeking, and it was developed empirically through Mary Ainsworth's observations of infant–mother interaction in the "Strange Situation," a procedure that classifies the quality of infant attachment into four categories.

**Secure attachment.** The child uses the caregiver as a secure base, explores freely when the caregiver is present, and is readily comforted. The caregiver is responsive and available.

**Insecure-avoidant attachment.** The child does not seek proximity and does not show distress on separation, and is self-soothing in the caregiver's presence. In the literature this pattern is associated with a history of unresponsive or rejecting caregiving, and the child's behaviour is understood as a suppression of the attachment system in the presence of a caregiver whose availability is unpredictable or whose response to distress is unhelpful.

**Insecure-resistant (or anxious-ambivalent) attachment.** The child seeks proximity but is not readily soothed, and is preoccupied with the availability of the caregiver. This pattern is associated with caregiving that is inconsistently responsive.

**Disorganized attachment.** The child displays contradictory, confused, or frightened behaviour in the presence of the caregiver, including approach and avoidance simultaneous with freezing. This classification was added by Mary Main and her colleagues and is associated with trauma, with maltreatment, and with unresolved loss or trauma in the caregiver's own history.

## 5.2 The developmental extensions

Three developments have extended attachment theory in ways that matter for adolescent practice.

**Internal working models.** Bowlby's proposal that the child constructs internal working models of the self and of others from the early attachment relationship has been developed into the most influential account of the adolescent's relational expectations. The adolescent who expects that others are unavailable will behave in ways that produce unavailability, and the practitioner who reads the adolescent's pushing away as evidence of a lack of investment is making exactly the error the theory predicts.

**The developmental transition at adolescence.** The developmental literature has established that adolescence involves a renegotiation of the attachment system in which the young person moves from the childhood state of dependence on caregivers toward a position of increasing autonomy, and that the process is observable in the changing quality of the parent–adolescent relationship, in the increasing salience of peers as attachment figures, and in the adolescent's increasing capacity to manage the attachment system without the caregiver present. The clinical implication is that parent–adolescent conflict is a normal feature

of this renegotiation, and that the question is whether the renegotiation is proceeding or has stalled.

**The peer attachment and the capacity for intimacy.** The developmental literature on peer relationships in the second decade establishes the increasing significance of peers as attachment figures, the developmental importance of the capacity to sustain a mutual, reciprocal relationship, and the association of the failure to develop that capacity with a range of later difficulties. The clinical relevance is that the peer relationships of an adolescent are a legitimate target of the work and not merely a distraction from it, and that a young person who has no friends is a young person with a developmental task uncompleted rather than with a social skill deficit.

### 5.3 The clinical use of attachment

The attachment literature is used in three ways in counseling practice.

**As a lens on the relationship.** A practitioner who understands attachment can interpret the young person's behaviour in the consulting room with much greater precision: the young person who cannot tolerate the practitioner's leaving the room, the one who is extremely compliant, the one who is testing and aggressive, the one who is charming and evasive, and the one who is indifferent are each displaying a particular attachment strategy, and each strategy is an attempt to manage a specific kind of expectation.

**As a framework for the assessment of caregiving.** The developmental and empirical literature on the parent–adolescent relationship has identified the features of caregiving that are associated with secure attachment in the adolescent years: warmth and responsiveness, a degree of autonomy-support, consistency, and a specific set of processes that have been called "child-centred" or "attachment-focused" parenting. Chapter 33 applies this to the practical task of working with parents.

**As a caution about causal claims.** The attachment literature contains a substantial number of correlational findings that are frequently reported as though they were causal, and a student should be cautious. The longitudinal findings are more robust than the cross-sectional ones, and the intervention studies — the video-feedback interventions with parents of infants and toddlers, and the growing body of work with parents of older children — are the appropriate basis for causal claims.

## 6. The Neurodevelopment of the Adolescent Brain

The adolescent brain is the most over-claimed topic in popular developmental psychology, and the student must be able to separate the evidence from the folklore.

### 6.1 What the evidence supports

**Continued myelination of the prefrontal cortex into the mid-twenties.** The prefrontal cortex, and particularly the regions responsible for planning, inhibition, and the evaluation of consequences, continues to myelinate into the early-to-mid twenties, and the grey matter in these regions peaks in early adolescence and is then pruned. The popular image of the adolescent as a "rational person with an un-rational accelerator" is a caricature, but it is a caricature of a real asymmetry.

**A relative acceleration of the socioemotional system.** The limbic and socioemotional regions of the brain mature earlier than the prefrontal regions, and the adolescent is consequently more responsive to social and emotional information, particularly to the emotional responses of others. This supports the observation that the presence of peers escalates both risk-taking and emotional arousal, and that peer relationships are

developmentally significant rather than incidental.

**The function of the adolescent's risk-taking.** The probabilistic reasoning research of the 1980s and 1990s, and the subsequent developmental neuroscience, indicate that adolescents do not differ from adults in their valuation of risk so much as in their perception of the uncertainty of outcomes, and in the increased influence of the potential for social reward. The adolescent's risk-taking is, on this account, a rational response to a rational assessment of a different environment, and the adult's prohibition of it is often an unexamined cultural preference rather than a response to a developmental fact.

**Sleep.** Adolescent circadian rhythms shift later during puberty, and the combination of a later phase with early school start times produces chronic sleep restriction in many adolescents, with measurable effects on mood, attention, and behaviour. This is a concrete and modifiable factor in the assessment of an adolescent with attentional or mood problems, and it is routinely overlooked.

## 6.2 What the folklore gets wrong

**"The adolescent brain is damaged"** — there is no evidence of damage, and the popular account of a rewired or malfunctioning brain is a misinterpretation of the normal developmental processes of myelination and synaptic pruning.

**"Teenagers cannot think about the future"** — the research shows a specific and interesting asymmetry. Adolescents are capable of sophisticated reasoning about the future in the abstract, and of long-term planning, and their deficit is specifically in the domain of the future that is uncertain, that is emotionally charged, and that is not continuously present. The practical implication for practice is specific and useful: the adolescent can engage with a long-term goal in the abstract, and cannot reliably engage with the goal in the presence of an immediate emotional pull. This is why a young person will agree enthusiastically with a plan on Tuesday and abandon it by Thursday, and it is why the plan's immediate steps matter more than its stated horizon.

**"Brain development explains behaviour"** — the neurodevelopmental account is used in the popular press and in some clinical writing as a determinative explanation, and it is not. Brain development describes a propensity; it does not explain a particular young person's decisions, and it is not a useful basis for declining to hold a young person responsible.

## 6.3 The clinical implications

Three implications are worth extracting, and they are the basis of the developmental modifications throughout this book.

**The environment must be structured rather than the intention strengthened.** Because the adolescent's capacity for self-regulation is genuinely immature in the presence of emotional arousal, the effective intervention is environmental: earlier bedtimes, a predictable timetable, planned responses, a parent who manages the transition home from school. The "just try harder" approach fails for reasons that are developmental, not motivational.

**Emotion precedes cognition in the moment of difficulty.** When an adolescent is emotionally aroused, the prefrontal capacity for reflective appraisal is reduced, and the cognitive techniques that depend on it are unavailable. The clinical consequence is that the cognitive work must be done when the young person is not in the middle of the difficulty, and the practice of pre-agreed coping strategies for the moment of difficulty is not a supplement to the cognitive work but a precondition for it.

**The peer context is developmentally significant.** Whatever the adolescent does in the presence of peers, they will do more of. The clinical implication is that working with a young person's peers, where this is possible and safe, is not a distraction from the individual work.

## Further Reading for This Chapter

- Damon, W. (Ed.). (1998). *Handbook of child psychology* (6th ed.). Wiley. — The reference work; use the individual chapters rather than the volume as a whole.
- Steinberg, L. (2014). *Age of opportunity: Lessons from the new science of adolescence*. Houghton Mifflin Harcourt. — The most readable and best-evidenced synthesis of the adolescent developmental literature.
- Sroufe, L. A. (2005). *The development of the person: The transactional origins of the self*. Guilford. — The definitive account of the attachment tradition's developmental claims, and the source of the caution about correlational findings.
- Rimm-Kaufman, S., & Jasch, R. (2013). *Social-emotional development in the school years*. — A practical developmental review aimed at the school context.
- Dahl, R. E. (2004). Pathways into and out of antisocial behavior. — A careful developmental account of the adolescent pathway into serious behavioural difficulty.

# 7. The Family Life Cycle and the Adolescent

## 7.1 The developmental position of the adolescent in the family

The adolescent occupies a distinctive structural position in the family system, and the structural family therapists of the 1970s identified this position with a precision that remains useful: the adolescent is the family member whose developmental task is the most clearly incompatible with the family's established organisation, and who is therefore the member most likely to become the location of the family's unresolved tensions.

The clinical observation is that families frequently organise around the adolescent's difficulty in ways that serve the system rather than the adolescent, and that the adolescent's symptoms, conduct, or withdrawal may be doing work for the family. Chapter 31 develops the structural account and Chapter 33 the practical work with parents.

Three consequences for practice follow.

**The apparent problem may be located in the wrong place.** A young person's refusal to attend school may be maintaining a marital conflict, a sibling rivalry, a grandparent boundary, or a parental avoidance. A practitioner who locates the problem entirely in the adolescent will spend the work on the wrong target.

**The developmental task is a family task.** The adolescent's push toward autonomy is a developmental task, and the family's response to it is a family process. The useful clinical question is not "how do I get this adolescent to comply?" but "what does this family do when one of its members wants to become an adult?"

**The risk of a too-early closure.** The structural clinicians described the developmental misfiling of the adolescent, in which a seventeen-year-old is placed in a role — the responsible one, the emotional caretaker of a parent, the surrogate parent of siblings, the holder of the family's emotional expression — that belongs to an earlier developmental stage. A good deal of what presents in adolescent practice is the consequence of a role that was allocated in the family long before the young person was asked to grow into it.

### 7.2 Family transitions

The family life cycle framework, in the formulation of Duvall, describes a sequence of developmental stages with characteristic tasks. The stage relevant here is the one in which the adolescent is leaving the family of origin and the young adult is establishing an independent household. Its tasks are the reorganisation of the marriage, the redefinition of the parent–child relationship, the development of the young adult's own household and occupational role, and the establishment of a satisfactory system of boundaries between the family of origin and the new household.

The clinical relevance of the framework is that it locates the young person's difficulty in a *transitional* period and makes the work of transition explicit, rather than treating the difficulty as a character trait. The 21-year-old who cannot leave home is not "dependent"; the 21-year-old who cannot leave home is in a transition whose tasks have not been accomplished, and the reasons are usually a mixture of developmental readiness, family system resistance, and the material conditions of housing and employment.

The framework also has a well-known limitation, which is that it describes an idealised sequence, with a nuclear family in a prosperous society, and that families in the present period — reconstituted, single-parent, transnational, multigenerational, in poverty, in exile — frequently do not pass through the stages in the sequence, and that the framework has been used historically to pathologise families that depart from it. The transitional understanding is nonetheless valuable when used descriptively rather than normatively.

## 8. Moral Development and the Adolescent's Ethical Reasoning

Kohlberg's three-level model of moral reasoning remains the most commonly used framework in the field, and its developmental sequence — the preconventional, the conventional, and the postconventional — has a straightforward clinical relevance: the level at which a young person reasons about a matter predicts the kinds of appeal that will work.

**The preconventional level** (approximately ages four to ten) is oriented to obedience and punishment and to the exchange of favours; the young person is oriented to what they will get and what they will avoid.

**The conventional level** (approximately ten to sixteen) is oriented to interpersonal approval and to law and order; the young person is oriented to what others will think and what the rules require. This is the level at which most adolescents are, and it is the level at which the appeal to "what would your friends and family think?" is developmentally appropriate and the appeal to abstract principle is not.

**The postconventional level** (approximately sixteen onwards, and reached by a minority) is oriented to social contract and to universal ethical principles.

The clinical use of the framework is specific. With a young person at the conventional level, a discussion framed in terms of principles will be heard as adult preaching; a discussion framed in terms of the effect on

the people they care about, or in terms of the agreement they themselves made, will be heard as relevant. And an intervention that appeals to a level of reasoning the young person has not reached will fail, however morally satisfying it is to the practitioner.

The framework's limitations are significant and should be acknowledged. The research on gender differences in moral reasoning that originally supported the model has been substantially undermined, and the sequence is now understood as less rigidly age-linked than Kohlberg proposed. And the model's individualistic and rights-based assumptions are not universal; the research on moral reasoning in non-Western settings finds a variety of structures, including a strong emphasis on relational and contextual reasoning, which the three-level scheme does not accommodate.

# 9. Social Development and Peer Relationships

## 9.1 The structure of the peer network in the second decade

The developmental research on peer relationships across the second decade establishes a clear sequence.

In early adolescence, roughly ages eleven to thirteen, there is a marked increase in the salience of peers, a decrease in the time spent with parents, and a rapid expansion and restructuring of the peer network, with the formation of larger and more complex groups, the rise of cliques, and the beginning of the developmental sequence of peer relations that moves toward the stable, reciprocal dyadic friendships of later adolescence.

In middle adolescence, roughly thirteen to sixteen, the developmental task of establishing a stable reciprocal relationship is prominent; the developmental literature identifies the capacity to sustain a mutual relationship, characterised by the ability to give and receive support, to manage conflict, and to sustain the relationship through periods of distance, as a key developmental competence.

In late adolescence, roughly sixteen to nineteen, the peer network contracts towards a smaller number of intimate relationships, and the developmental significance shifts towards the capacity to sustain a committed relationship over time, with a concomitant decline in the pressure of the peer group.

## 9.2 Why the peer relationship is a clinical target

The developmental significance of the peer relationship for an adolescent explains why peer relationships are a legitimate target of the therapeutic work, and why their absence is a developmental task uncompleted rather than a peripheral deficit.

The evidence linking peer connectedness and quality of close relationships to a wide range of adolescent outcomes is substantial, and it runs in both directions: the quality of the adolescent's relationships is associated with lower rates of depression, anxiety, substance use, and suicidality, and with better academic engagement, and the direction of the causal relationship between peer relations and internalising symptoms is itself complicated by the fact that depressed and anxious young people are, on the one hand, more likely to experience peer difficulties and, on the other hand, more likely to withdraw from peers in a way that is then read as a preference.

The clinical implication is that a young person whose peer relationships have collapsed is a young person for whom a central developmental task has become blocked, and that the work of restoring the possibility of peer connection — which usually means restoring the conditions for it, rather than instructing the young

person in social skills — is a legitimate and often central part of the treatment.

### 9.3 The limits of social skills training

The social skills training literature is large, and its evidence base for children with conduct problems is reasonably good. Its application to adolescents with peer withdrawal is more problematic, for three reasons.

The first is that skills training presupposes that the deficit is in skills, and in many cases of adolescent withdrawal it is not. The young person may have the skills and not the opportunity, or may have the skills and not the motivation, or may be correctly assessing a social environment that does not offer a safe place to be a friend. A skills programme delivered in the second case teaches a young person to approach people who have already shown they are not safe to approach.

The second is that the research on the generalisation of social skills training is not encouraging: the skills are frequently learned in the training context and do not transfer to the natural environment.

The third is that the approach can be experienced as a further instance of the adult world telling the young person what is wrong with them and prescribing a performance of normalcy, and for an adolescent whose withdrawal is a deliberate response to a social world that has been hostile, the prescription is both ineffective and ethically questionable.

## 10. Developmentally Appropriate Formulation

The developmental perspective transforms the formulation in five specific ways, and a student should be able to name them for any case.

**The developmental description.** The formulation should state what is developmentally expected at the young person's stage, in the specific domains in question, and what the deviation consists of. "She is avoidant of unfamiliar peers in a way that exceeds the normal caution of early adolescence" is a developmental statement; "she has social anxiety" is a classificatory one that does no work.

**The developmental task.** The formulation should identify the developmental task that is blocked, in the young person's own terms. For an adolescent with a peer collapse, the task is the establishment of a stable reciprocal relationship; for a fifteen-year-old who is sexually active and is frightened of a pregnancy, the task is the integration of a new sexual and relational identity; for a nine-year-old who is failing at school, the task is the acquisition of the skills of industry and the sense of competence.

**The developmental mismatch.** The formulation should identify where the young person's current development and the demands being made of them are out of alignment. The most common such misalignments in practice are the school that demands adult-level self-regulation from a fourteen-year-old; the family that demands adult-level emotional responsibility from a twelve-year-old; and the service that demands adult-level insight from a nine-year-old.

**The developmental trajectory.** The formulation should state what the direction of change is likely to be with or without intervention. This is the most clinically useful and the most neglected element, because developmental trajectory is the thing that tells a practitioner whether to wait.

**The developmental timing of the intervention.** The formulation should state why now is the time to intervene and why the specific intervention is appropriate at this developmental stage rather than earlier or later. A family-based intervention with parents of a fourteen-year-old's peer withdrawal may be better

delivered at fifteen, when the developmental task becomes more salient.

# 11. Developmental Assessment

## 11.1 Sources of developmental information

Developmental assessment draws on five sources, and a practitioner should systematically consider all five rather than relying on the young person's self-report.

**The young person's account.** Valuable, developmentally biased in predictable ways, and subject to the distortions discussed in the cognitive chapter. A young person at the concrete operational stage will report concrete events accurately and will be unreliable about abstract, general, or future-oriented questions, and a practitioner who asks "how often do you usually feel sad?" of a nine-year-old will receive an uninterpretable answer.

**The parent's account.** Informed by a longer time span and by the parent's own developmental expectations, and shaped by the factors that make parent accounts unreliable — the parent's own unmet developmental needs, the difficulty of a parent's appraisal of an adolescent's internal state, and the tendency to attribute to the young person what is actually a family process.

**The teacher's account.** Informed by a different context and a different set of observations, and usually focused on the visible and the disruptive. A teacher may be an excellent observer of a young person's social position and an unreliable observer of their internal state.

**Records and previous assessments.** Often the only source of information about early development, and frequently available where the young person's own account is not. Chapter 53 discusses the assessment instruments and their use.

**Direct observation.** The only way to establish what actually happens, and under-used in practice. Where it is possible to observe the young person in a peer setting, in the classroom, or with a family member, the observation is frequently more informative than any account.

## 11.2 The developmental interview

A developmentally appropriate developmental interview has five components, and the ordering matters.

**The current functioning.** What the young person is doing now, in concrete terms. The anchor of the whole interview, because it is the most reliable data and because it establishes the base rate.

**The history.** The sequence of significant events, in order, with attention to the timing in relation to the developmental periods. The developmental significance of an event is a function of when it occurred, and the practitioner should be reconstructing the timeline explicitly.

**The developmental tasks.** What the young person is currently working on, in the domains of the framework: identity, intimacy, autonomy, industry, and so forth. Asking about a task in a developmentally appropriate way requires the abstract language of the framework and the concrete language of the young person.

**The caregiving environment.** Who is in the household, who does what, how the young person's care is arranged in practice rather than in intention, and what the history of the family has been.

**The young person's own account of their development.** How they see themselves changing, what they are trying to become, what they are afraid of becoming. This is the most clinically useful part of the interview and the part most often omitted, because it requires a level of trust that takes more than one session.

### 11.3 The developmental formulation template

The template in Table 9.1 is a useful working structure for a developmental formulation and is used in the formulation assignment described in Chapter 58.

| Element | Question it answers |
|---|---|
| Developmental stage and its tasks | What is expected at this stage and what is the young person working on? |
| Developmental description | What is observed, in what domains, and how does it depart from expectation? |
| History and timing | What happened, and when, and what is the developmental significance of that timing? |
| Caregiving and context | Who cares for the young person, and how, and with what history? |
| Current maintaining processes | What is keeping the difficulty going, at each level of the system? |
| Developmental trajectory | Where is this heading with and without intervention? |
| Developmental appropriateness of the plan | Why this intervention, and why now? |
| Review points | When will the developmental trajectory be re-examined? |

## 12. Case Illustration: The Child Who Could Not Be Silly

**The referral.** Ruben, nine, is referred by his school and his mother for "anxiety." He has not attended school for eleven weeks. He had been markedly shy in his first year and had one friendship, which ended in the summer before the referral. He is bright, does his homework, reads voraciously, and his mother reports that he "is perfectly fine at home — he's just got on with his games and his books and he's not interested in people."

**The developmental questions.** The practitioner, working from the developmental framework, asks a series of questions that are the same questions one would ask in an adult case and that are worth spelling out, because the answers change the case entirely.

**What is expected at nine?** Industry versus inferiority. The developmental task is the acquisition of the skills of the school subjects and the sense of competence that comes from doing them well. This is the year in which the child learns that he is good at some things and not others, and the sense of "I am a person who can do some things" is established.

**What happened, and when?** The friendship ended in the summer. Ruben's mother, a single parent working shifts, describes the summer as "fine" and then concedes that she was working eighteen-hour shifts and that Ruben's grandmother, who had been his daytime carer, moved to her daughter's house in another town in May. The single most important fact in the case arrives at the third interview.

**What does Ruben do with people?** He does not know how to answer the question. When pressed, he says that he "can talk to people about things" — and then, after a long pause, that he has thought about what to say first, and that he has not been able to think of anything. The practitioner recognises this as a description of a failure of the social-cognitive problem solving that the developmental literature describes as central to the peer task in early adolescence and important already in the school-age years.

**What is the caregiving environment?** Ruben's mother is present but exhausted, and the practical arrangements are that Ruben is largely alone in the evenings.

**The formulation.** The developmental formulation is that Ruben is a child who has lost, in the space of two months, the peer relationship that was the site of his developmental task, and the caregiving arrangement that supported it, and has retreated into the one domain in which he is unambiguously competent — his reading and his games. The anxiety is not a primary emotional disorder; it is the developmentally expectable response to the loss of the social context in which a nine-year-old's industry can be exercised, in a child who has no other domain in which to be competent, and who has lost the adult who was available.

**Why the standard approaches would have failed.** A behavioural approach built on graded social exposure would have required Ruben to approach other children, and the reason he does not is that he has not yet worked out how to begin a conversation. A cognitive approach would have required him to produce a thought record, and at nine, in the concrete operational stage, the abstract propositional thought required is at the limit of what he can do, and in any case the problem is not a distorted appraisal. A family approach would have been the right first move in any case, and it is complicated by the fact that his mother's shifts are not a choice she has made freely.

**What the practitioner does.** The first intervention is practical: the shifts are renegotiated, the grandmother's departure is acknowledged, and the school is asked to accommodate a return with a specific adult peer to talk to, because the developmental research is clear that peer-supported rather than adult-supported re-entry is what works with young children. The second intervention is to the developmental task itself: Ruben's competence is extended from the solitary domain into a social one, through a structured, small-group, play-based activity with three other children, run by a practitioner who is explicitly a play therapist. The third is the developmental check: what happened to the sense of competence that he has in reading, and can it be generalised?

**The teaching point.** The referral said "anxiety." The developmental formulation says "loss of the site of the developmental task, with a predictable developmental response." The second is a formulation that predicts a course and identifies an intervention; the first is a label that predicts nothing and generates a treatment that will not work.

## 13. Evidence and Research Support

**Well supported.** The association between supportive adult relationships and adolescent outcomes. The association between peer connectedness and quality and adolescent mental health. The developmental sequence of peer relationships across the second decade. The increase in peer salience and the shift in the parent–adolescent relationship across adolescence. The normative progression of cognitive capacity and the specific limits of abstract reasoning in the school-age years. The association of sleep restriction with adolescent mood and attentional difficulties. The relationship of attachment classification in infancy to a range of later relational and emotional outcomes, with the magnitude of the associations being modest and the causal claims being appropriately cautious.

**Supported with qualifications.** The claim that the prefrontal cortex continues to develop into the mid-twenties and that this has behavioural consequences; the anatomical data are well established and the behavioural inferences are less direct than the popular presentation implies. The claim that the adolescent brain is "prone to risk"; the risk-taking findings are robust and the explanation of them is contested. The claim that identity exploration is the central developmental task of adolescence; the framework is theoretically influential and empirically weaker than its popularity suggests. The claim that moral reasoning develops in the Kohlbergian sequence; the sequence has partial support and the specific predictions are weakly supported.

**Weakly supported or contested.** The developmental appropriateness of the stage boundaries. The claim that the person is fully capable of insight at a given age. The claim that early attachment predicts later adjustment with the precision usually implied. The universal applicability of the nuclear-family developmental sequence.

**Absent or very thin.** Evidence on the developmental adaptation of specific therapies, which is generally limited to a small number of well-studied approaches. Research on the effectiveness of developmentally adapted interventions with specific populations. Longitudinal research linking specific developmental tasks to specific intervention components.

## 14. Strengths and Limitations of the Developmental Perspective

**Strengths.** It replaces moral judgement with interpretation, and that substitution is the single most valuable thing the perspective offers a practitioner working with adolescents. It constrains technique selection in ways that improve outcomes. It provides a framework for prognosis and for timing, which are the aspects of practice most often handled by habit. And it supplies the developmental task as a concrete, non-pathologising description of what a young person is trying to do.

**Limitations.** Its normative data are old, Western, and derived from unselected samples, and the norms have shifted considerably since they were collected — which means that a considerable part of the "developmental" knowledge in the field is in fact historical. Its stage models are more rigid than the data support, and the risk of a practitioner treating a young person as "too young" for a task is a real one. The perspective is individualistic, and it sits awkwardly with the ecological and systemic approaches. And the enormous research literature on the second decade has been generated in cultures whose adolescence looks unlike that of the cultures many of the field's practitioners work in.

## 15. Cultural and Ethical Considerations

**Development is culturally organised.** The developmental tasks, their timing, and their content are not universal. The developmental literature on schooling shows that the age of formal secondary education, the content of the curriculum, and the age at which adult-like responsibilities are assumed vary enormously across societies, and the adolescent in one context may be functioning at a developmental level that the adolescent in another context reaches several years later. The developmental framing in this chapter is a Western developmental framing, and the practitioner should treat it as one set of expectations among several.

**The danger of developmental explanation of harm.** The developmental perspective can be misused to explain away the harm a young person has experienced — to treat the adult's failure as a developmental stage, or to treat the consequences of poverty or discrimination as a maturational lag. A practitioner who uses the developmental lens to soften the description of a young circumstances will find that the softening is itself the harm. The developmental perspective should describe what the young person is doing; it should not describe what has been done to them.

**The ethics of developmentally appropriate pacing.** The requirement that interventions be developmentally matched carries an ethical obligation that is not always recognised: that the young person should be treated in a manner appropriate to their age in matters of privacy, explanation, decision-making, and confidentiality, and that the explanation of the treatment given to a young person should be in language the young person can actually use. A nine-year-old who is given a "confidentiality" explanation designed for a forty-year-old has been given less than they were owed.

**The ethical requirement of developmental assessment.** The ethical codes require assessment that is appropriate to the client's developmental capacity, and that requirement is not satisfied by the mere administration of an adult instrument to a child. The assessment of a young person should be developmentally appropriate, should gather information from multiple sources, and should be reviewed in light of what the young person can contribute.

## 16. Key Terms

### Key Terms at a Glance

- **Developmental appropriateness** — The requirement that an intervention be matched to the young person's cognitive, emotional, social, and autonomy capacities; a condition of validity rather than an accommodation.
- **Zone of proximal development** — The distance between what a child can do independently and what she can do with guidance from a more knowledgeable other; the basis for the optimal difficulty of a task.
- **Scaffolding** — The provision of temporary support that is progressively withdrawn as the young person becomes capable; a description of both the learning process and the developmental shape of the counselling relationship.
- **Concrete operational stage** — The developmental stage from approximately seven to eleven, in which logical reasoning is available for concrete material but not for abstraction.
- **Formal operational stage** — The stage from approximately eleven onwards, in which abstract and hypothetical reasoning becomes available, unevenly and in a context-dependent way.
- **Decentration** — The developmental capacity to consider more than one aspect of a situation simultaneously, and to distinguish appearance from reality; acquired in the concrete operational stage.
- **Executive function** — The set of capacities — working memory, inhibitory control, cognitive flexibility, and planning — that support goal-directed behaviour; a key target of developmental assessment with young people.
- **Working memory** — The limited-capacity system for holding and manipulating information; a common bottleneck in the cognitive techniques with young people.
- **Identity foreclosure** — A premature and rigid commitment, often made under family or community pressure, that forecloses further exploration; the developmental form of difficulty in the adolescent identity task.
- **Possible self** — The version of oneself one is trying to become, together with the feared version; central to the developmental account of adolescent anxiety and depression.
- **Attachment** — The early developmental system of proximity-seeking and security; the foundation of the account of relational expectation and of the individual differences in it.
- **Internal working model** — The enduring representation of the self and of others formed in the early attachment relationship and carried forward into later relationships.
- **Strange Situation** — Ainsworth's procedure for classifying the quality of infant attachment into secure, avoidant, resistant, and disorganised patterns.

**Key Terms at a Glance (continued)**

- **Disorganized attachment** — The attachment pattern associated with trauma, maltreatment, and unresolved loss in the caregiver, marked by contradictory or frightened behaviour in the presence of the caregiver.
- **Parental autonomy support** — The caregiving characteristic of providing rationale, choice, and responsiveness rather than control; associated with better adolescent outcomes.
- **Peer attachment** — The developmental significance of peers as attachment figures in adolescence, and of the capacity to sustain a reciprocal relationship.
- **Eveningness chronotype** — The developmental shift in circadian timing during puberty, and the source of the mismatch with early school start times.
- **Possible risk sensitivity** — The adolescent tendency to weight the potential for social reward more heavily, and uncertainty less accurately, than adults do.
- **Developmental trajectory** — The expected course of the young person's difficulty with and without intervention; the basis of prognosis and of the decision whether to wait.
- **Developmental mismatch** — The misalignment between the demands made of a young person and their developmental capacity in that domain.

## 17. Chapter Summary

Every theory in this book was developed with adults, and the transfer of an adult theory to a young person requires developmental modification in the areas of interpretation, technique selection, dosage, and meaning. The developmental perspective is a discipline of interpretation that replaces moral judgement with a developmental account of what the young person is doing.

The frameworks in this chapter contribute different things. Piaget's stages and Vygotsky's zone of proximal development constrain technique selection and establish the optimal difficulty of a task. Erikson's psychosocial tasks, and the more recent work on the adolescent identity task and the possible self, describe what the young person is working on. The attachment tradition explains relational expectations, the parent's role, and the developmental significance of the adolescent's separation from the family. The neurodevelopmental literature explains the adolescent's heightened sensitivity to social and emotional information, the delayed maturation of self-regulation, the shift in circadian timing, and the specific asymmetry in the capacity to reason about an uncertain and emotionally charged future.

Two domains are easy to overlook and central in practice: the family life cycle, which locates the adolescent's difficulty in the family system and identifies the developmental task as a family task, and the peer relationship, which is a developmental task in its own right and a legitimate target of the work.

The developmental formulation specifies the stage and its tasks, the developmental description, the history and its timing, the caregiving and context, the maintaining processes, the trajectory, the developmental appropriateness of the plan, and the review points. The case of Ruben illustrates the whole argument: a referral that said "anxiety" was reformulated, through the developmental questions, into a loss of the site of the developmental task with a predictable developmental response — a formulation that predicted a course

and identified an intervention.

## 18. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the difference between the developmental perspective as interpretation, as selection, as dosage, and as meaning, using a single example for each.
2. State the clinical limitation of a thought record with a nine-year-old, and explain it in terms of the concrete operational stage.
3. Apply the zone of proximal development to the design of a graded task for a fourteen-year-old avoiding school.
4. Distinguish the adolescent identity task from identity foreclosure, and describe a case of each.
5. What does the attachment literature predict about a young person who pushes their practitioner away? What is the likely error of interpretation?
6. State three claims about the adolescent brain that the evidence supports, and three popular claims that it does not.
7. Explain why the statement "she is too young for insight" is often wrong.
8. What does the developmental perspective contribute to the decision about whether to intervene now or wait?
9. Describe the developmental task that is blocked in the case of Ruben, and explain why the standard approaches would have failed.
10. In what sense can the developmental perspective be misused to explain away harm, and what is the ethical requirement that prevents this?

## 19. Practice Activities

**Practice and Application**

### Activity 9.1 — The Developmental Timeline (2 hours, individual)

Take a case from your placement or a peer role-play and construct a developmental timeline: a single vertical axis, aged zero to eighteen, on which every significant event, both developmental and historical, is placed. Then draw a second axis, marked with the developmental tasks, and overlay it. In three hundred words, identify the two points at which the historical event and the developmental task interact. This exercise is the fastest way to learn to see a case developmentally, and it is directly transferable to the formulation assignment.

### Activity 9.2 — The Same Technique, Three Ages (90 minutes, groups of four)

Each group takes a single technique from this book — thought recording, exposure, behavioural activation, or the miracle question — and specifies exactly how it would be delivered to a seven-year-old, a twelve-year-old, and a seventeen-year-old: the wording, the medium, the session length, the presence of the parent, the homework, the review points, and the expected timeline. The differences between the three versions will be larger than most students expect, and the exercise is the best preparation for the developmental adaptations described in the technique chapters.

### Activity 9.3 — The Developmental Interview (75 minutes, triads)

Conduct a developmental interview with a peer playing a fourteen-year-old who has recently lost a friend and has withdrawn. Use the five components in Section 11.2 and the five assessment questions in Section 11.1. The observer records which questions produced the most useful information and which produced confusion. The purpose is to establish the developmental history in a way that is developmental rather than an interrogation, which is a skill that takes practice and is not taught anywhere else.

## 20. Annotated Reading Pathways

**For the developmental science.** Read Steinberg (2014) for the adolescent synthesis, the relevant chapters of Damon (1998) for the primary sources, and Sroufe (2005) for the attachment tradition. Read them critically: the normative data are old and the field's confidence exceeds its data.

**For the clinical application.** Read the developmental chapters of the child and adolescent psychotherapy handbooks, and then read the "developmental considerations" sections in each technique chapter of this book as a set. Read them together, because the pattern of what is required is clearer when the chapters are read consecutively.

**For the neurodevelopmental claims.** Read the review literature rather than the popular accounts, and be especially careful with the language: "the adolescent brain is not fully developed" is a statement that is both true and misleading, and the group of students who can explain precisely in what sense it is misleading will be at a professional advantage.

**For the cultural critique.** Read the developmental literature outside the United States and Western Europe, and the cross-cultural developmental psychology literature on the cultural organisation of the second decade. The developmental expectations in this chapter are a specific cultural product, and a practitioner who works with a different population should be able to say so.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Developmental Foundations for Counseling Youth** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the developmental science that constrains and informs any theory applied to children and adolescents. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The mechanism to keep in view is how a theory turns observations into explanations, predictions, interventions, and reviewable outcomes. The characteristic professional risk is mistaking an elegant explanation for evidence, or applying a preferred theory before the young person has been understood. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in the first counselling sessions, assessment meetings, and case discussions, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Leah is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Leah's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Leah explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on conceptual clarity, evidence appraisal, therapeutic relationship, and disciplined formulation. The first intervention is deliberately modest. Leah chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Leah has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Leah can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Developmental Foundations for Counseling Youth

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Developmental Foundations for Counseling Youth** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Developmental Foundations for Counseling Youth** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Developmental Foundations for Counseling Youth**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Developmental Foundations for Counseling Youth to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART II

# Person-Centered and Humanistic Theories

Theories that locate the person, not the problem, at the centre of change: the growth-oriented traditions inaugurated by Carl Rogers and extended by existential, Gestalt, phenomenological, and dialogical thinkers.

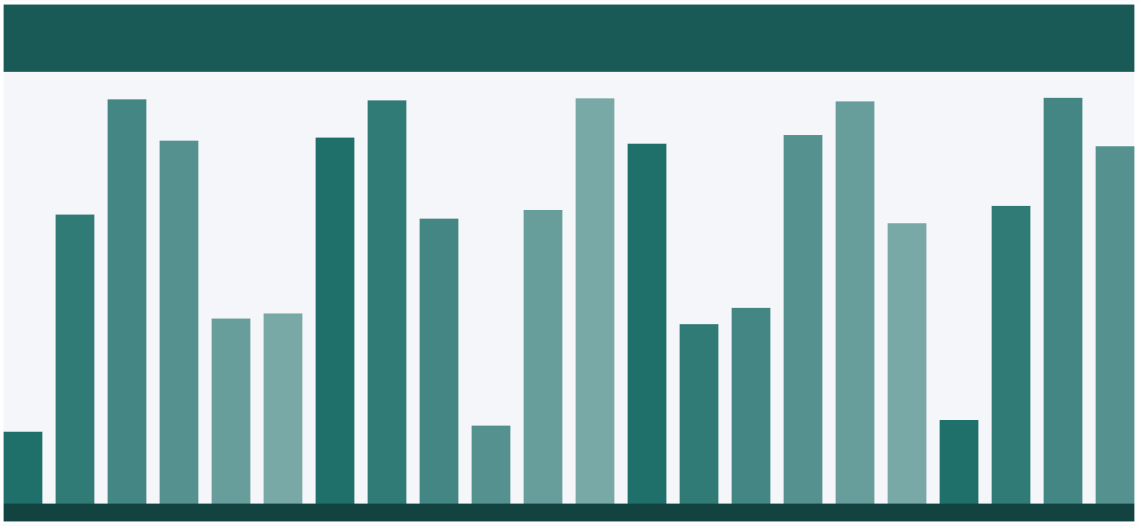
## CHAPTERS IN THIS PART


10 Person-Centered Theory: The Client as Expert
11 Existential Therapy: Meaning, Isolation, and Freedom
12 Gestalt Therapy: Awareness, Contact, and the Here and Now
13 Focusing-Oriented Therapy and the Felt Sense
14 Transactional Analysis: Structure, Strategy, and Script
15 Internal Family Systems and Structural Dissociation

CHAPTER 10

# Person-Centered Theory: The Client as Expert

Rogers' theory of the self, the actualising tendency, and the conditions of therapeutic change.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the historical and philosophical context of person-centred theory, and identify the four influences on Rogers' thinking.
- State the propositions of Rogers' theory of personality, including the self-concept, the organismic experience, and the fundamental needs.
- Explain the concept of incongruence and describe the process Rogers proposed for the reduction of incongruence.
- State the necessary and sufficient conditions of therapeutic personality change and explain what "necessary" and "sufficient" mean in this context.
- Describe the core therapeutic conditions and give a concrete clinical example of each.
- Apply person-centred theory to a case involving a child or adolescent, and identify what the theory requires and what it makes difficult.
- Describe the modern developments in person-centred theory, including the focus on the person as a process, the integration of affect and cognition, and the social and political dimensions.
- Evaluate the evidence for person-centred therapy and for the therapeutic alliance as the common factor.

## Chapter Overview

Carl Rogers' client-centred theory was published in 1942 and revised in 1951, and it is one of the very few theories in counseling that has remained continuously in clinical use for eight decades. Its longevity is remarkable and its reasons are instructive: it was the first theory in the field to give the practitioner a testable account of the conditions under which change occurs; it was the first to make the relationship itself the object of theory; it was the first to be written in the language of the client's own experience, which made it immediately usable by practitioners without a research background; and it produced the most replicated finding in the field's history, the relationship between the quality of the therapeutic alliance and outcome.

This chapter presents the theory in its full structure. It begins with the context, in which the personal and intellectual situation of its author and the professional situation of the field in the 1940s are both relevant, and it includes the episode of Rogers' break with the psychoanalytic movement, which was the first public act of theory-making in the profession and set a pattern for the conflicts that followed.

The chapter then works through the theory's propositions: the person as an organism with an innate actualising tendency; the self-concept and the internal frame of reference; the distinction between the organismic experience and the self-concept; the fundamental needs; and the process of change, which Rogers described with an accuracy that remains remarkable.

The final third of the chapter addresses three practical questions: what person-centred theory requires of a practitioner working with children and adolescents, which is a harder question than it is usually treated as; what the evidence says; and what the modern developments in the theory, including the attention to power and to social conditions, add to the original formulation.

## 1. Historical and Philosophical Context

## 1.1 The four influences

Rogers' theory is usually presented as emerging from a single clinical tradition, and it is more accurate to present it as the convergence of four.

**The clinical method of Otto Rank and the Rankian "will" psychology.** Rogers' first clinical work was as a psychologist at a mental hospital in Ohio, where he was a member of a team following Rank's approach. From it he took the emphasis on the *will* — the striving of the person, not merely the reaction of the person — and the emphasis on the creative, forward-moving quality of human behaviour. Rogers' formulation of the actualising tendency is the direct descendant of Rank's will.

**The field theory of Kurt Lewin.** Rogers' doctoral work was in Lewin's group, and he inherited the field theory's central proposition: behaviour is a function of the person and the environment, understood as a field in which the person is located, rather than as a reaction to a stimulus. The phrase "person and environment" appears in Rogers' formulations in a way that reveals the lineage, and the entire framework is a person-in-field theory in Lewin's sense.

**The phenomenological tradition.** Rogers' college training included a year of study in religion and philosophy, and he was decisively influenced by the phenomenology of the European tradition, transmitted through the American reception of Brentano and the work of the Chicago school. The consequence is the client-centred theory's central methodological commitment: that the only authority on a person's experience is that person.

**The psychodynamic tradition, from which he broke.** Rogers' work at the Menninger Clinic in the 1930s exposed him to psychoanalytic and neo-Freudian practice, and the experience of the analytic relationship — which Rogers described as an "inescapable" and often puzzling encounter — is the direct source of the client-centred method's central concern with the practitioner's own congruence.

## 1.2 The break with psychoanalysis

The episode of 1939, in which Rogers presented a paper at the annual meeting of the American Psychological Association challenging the practice of psychoanalytic interpretation, is the founding moment of the theory and of the profession's reputation for theoretical conflict. Rogers' argument was that the analyst's interpretations were a form of advice, and that the analytic relationship had no clear goals, no responsibility, and no accountability; and that the analyst, in effect, "reframed" the client's experience according to the analyst's own frame, which was a "schizophrenic" withdrawal from the client's actual frame of reference.

The paper was received as an attack on the psychoanalytic establishment, and it cost Rogers his position. The episode matters to the present-day student for two reasons. First, it is the origin of the professional distinction between the client-centred and the psychoanalytic traditions, and the reasons for the distinction are still the best short statement of the difference between them. Second, it is the origin of the profession's tradition of theoretical debate conducted in public and with considerable personal cost, which is a tradition with a mixed record and which the student should approach with a critical eye.

## 1.3 The professional context

Client-centred theory appeared at a moment when the dominant professional model was the medical one, in which the practitioner was an expert in a disorder and the client was a passive recipient of an expert service. Rogers' theory inverted every element of that model: the practitioner was not an expert in disorders but a

facilitator of a process; the client was not a patient but the expert on their own experience; the goal was not the cure of a symptom but the resolution of a discrepancy within the person; and the theory of the practitioner became a part of the theory of the process.

The subsequent history of the theory is a history of the professional adoption of these commitments, which is why client-centred theory is now less a distinct therapy than a set of commitments that pervade the field. The codes of the profession, the measurement instruments (the Client Outcome Questionnaire, the Session Rating Scale), the emphasis on the alliance, the practice of routine outcome feedback, and the widespread use of the term "client" rather than "patient" are all descendants of Rogers' argument.

# 2. The Theory of the Person

## 2.1 The organismic experience

Rogers' account begins with what he called the **organismic experience** — the total, ongoing, ever-changing stream of experience that constitutes the person's actual life, comprising three distinguishable elements: the organism's experience of itself, of the environment, and of the relationship between the two. This is a phenomenological, not a neurophysiological, claim. Rogers is describing the person from the inside, and the description is of a live, moving, purposive process.

Three characteristics of the organismic experience are critical.

**It is always changing.** The organismic experience is a process, not a thing, and at every moment the whole of the person's experience is reorganising in the direction of its fulfilment, subject to the conditions described below.

**It is available to awareness.** The organismic experience is, in principle, fully available to the person's awareness, although the accuracy with which it is available varies enormously and is the key to the theory's account of maladjustment.

**It is governed by the actualising tendency.** This is the deepest claim. The organism, in its functioning, moves toward the fulfilment of its needs, and the fulfilment of the actualising tendency is a consequence of the organisation of the whole experience toward the coherence and integration of the organism's experience and the self. The actualising tendency is not a drive to be satisfied like a hunger; it is a tendency in the organisation of experience, and it is described in process terms rather than in need terms.

## 2.2 The self-concept

The **self-concept** is Rogers' term for the organism's internal representation of itself — the set of perceptions, judgments, and feelings that the person holds about who they are, which is accessible to awareness and which is organised in a way that is consistent with the person's experience.

The critical feature of the self-concept is that it is **incomplete**. It does not and cannot encompass the whole of the organismic experience, because it is a symbolisation, and symbolisation is always selective. There are two consequences. First, experiences that are inconsistent with the self-concept are denied symbolisation and are therefore not available to awareness. Second, and more consequentially for the therapy, the self-concept operates as a **selection apparatus**, admitting into awareness only those experiences that are consistent with it, and thereby determining what the person can learn about themselves.

This is the theoretical core of the entire theory, and the student should understand it well: **incongruence is not simply a discrepancy between two things; it is a discrepancy that operates as a filter on the person's access to their own experience.** A young person whose self-concept includes "I am the one who holds everything together" cannot fully experience the fact that they are exhausted, because the exhaustion is inconsistent with the self-concept, and the self-concept will not permit it into awareness. And the exhaustion will then be expressed behaviourally, which is exactly what the parent reports.

### 2.3 The fundamental needs

Rogers identified four basic psychological needs. The third of these is the most famous and the most often quoted out of context, and the other three are essential to the theory.

**The need for positive regard.** The need to be regarded by others as a person with worth, and to regard oneself in the same way. It is not a need for approval, and it is not a need to be liked; it is a need to be *accepted as a person*, which is why the "unconditional" qualification is decisive.

**The need for affection.** The need for some form of tender, caring response from others. It is a need for the experience of being valued and cared for, and its frustration produces a sense of worthlessness.

**The need for inclusion.** The need for the experience of being included in a group or a relationship, and the need to have an effect on the people with whom one is connected. Its frustration produces a sense of isolation and of being a stranger.

**The need for congruence.** The need for one's own behaviour, and one's own experience, to be in accordance with one's self-concept. It is a need for authenticity, for the sense that what one is and what one does and what one feels are one and the same thing. Its frustration produces anxiety, and it is the need whose frustration Rogers' theory locates at the centre of maladjustment.

### 2.4 The conditions of growth

The actualising tendency operates, in Rogers' account, under three conditions, and all three must be present for growth to occur.

**The psychological condition of congruence.** The person must be in a state in which the experience is fully congruent with the self-concept — which is to say, in which the person is in contact with their own experience.

**The psychological condition of unconditional positive regard.** The person must experience the *other* as understanding and accepting the self as it is, without conditions of worthiness.

**The psychological condition of empathic understanding.** The person must experience the other as understanding the person's experience from the inside.

Rogers' account is that the actualising tendency operates in the individual, in the family, and in the group, and that its operation in each depends on the presence of these three conditions. The implication for the therapy is direct and is the most important single proposition in the theory: **these three conditions are also the conditions of therapy**. The conditions in which the person grows are the conditions the practitioner must create, and nothing else is required of the practitioner for growth to occur.

## 3. The Theory of Maladjustment

### 3.1 The process of incongruence

Maladjustment, in Rogers' account, is not a set of symptoms and not a disease. It is a process with a specific structure, and the structure is the following.

An experience occurs that is inconsistent with the person's self-concept — because the experience is threatening, because it would require a major reorganisation of the self-concept, or because the person has never learned a way of symbolising it.

The experience cannot be symbolised into the self-concept without distortion. Two things follow. First, the experience is denied access to awareness, and the person becomes out of touch with it — which is the origin of the anxiety. Second, the inconsistent experience is expressed in behaviour that is distorted, in the sense that it expresses the underlying need in a form that is incongruent with the self-concept and is consequently ineffective or maladaptive. The characteristic example is the aggressive or destructive behaviour of a person whose self-concept includes "I am a loving and gentle person" and who has learned to express anger only in forms that preserve the self-concept.

The distortion of need into maladaptive behaviour is the theoretical heart of the account, and it has a number of important clinical implications. It explains why a young person's problem behaviour is often *the best available option within their self-concept*, and why telling them that they "could behave differently" is not only unhelpful but theoretically incoherent. It explains why a young person who is intelligent and capable can be extraordinarily clumsy with their own feelings. And it explains why the therapeutic task is not the elimination of the maladaptive behaviour but the restoration of the person's contact with the experience beneath it — because once the experience is available to awareness, the actualising tendency will begin to organise it more effectively, and the distorted expression will no longer be required.

### 3.2 The concept of the "real self"

Rogers' term for the person's underlying, organising experience — the source of the actualising tendency, and the potential for a fuller functioning that is experienced in the person but not symbolised — is the "real self" or the "self" in the process sense, as against the self-concept. The "fully functioning person" is one in whom there is a close correspondence between the organismic experience and the self-concept, so that the person is in touch with their own experience and able to act from it.

The therapy is the reduction of incongruence, and the goal is not to install a better self-concept but to remove the filter that prevents the person from having their own experience. The subtlety here is worth dwelling on, because it is the point at which Rogers' theory is most frequently and most seriously misunderstood: **the aim is not to make the person into what they should be, or what the practitioner thinks they could be, or what their potential allows. The aim is to make it possible for the person to be who they already are, without the distortion.** This is why the theory is a theory of the client as expert, and why the practitioner's imposition of an alternative identity is a violation of the theory's central claim.

## 4. The Necessary and Sufficient Conditions of Therapeutic Personality Change

The 1957 paper in which Rogers set out these conditions is, in the judgement of many in the field, the most important paper in the twentieth-century counseling literature, and it is worth working through its argument closely.

Rogers' claim is that certain conditions, if present in a counselling relationship, are both **necessary** and **sufficient** for personality change to occur. The conditions are the three already described — congruence, unconditional positive regard, and empathic understanding — with the addition of the structural conditions: that the two parties are in psychological contact; that the client is incongruent; and that the client is capable of, and motivated to, a process of change.

The claim of sufficiency was and remains the most contested part of it. Rogers himself later qualified it, saying that the conditions are sufficient to *initiate* a process of change and that the process itself then proceeds by the person's own actualising tendency, and that the conditions are necessary throughout for the process to continue. The logical objection is straightforward: the claim that a set of relational conditions is sufficient for a specific outcome invites the counter-example of a client who is not yet ready, or who does not want change, or whose circumstances make change impossible. Rogers' answer — that such a client does not meet the sixth condition, of motivation — is elegant and, in practice, unfalsifiable, since the motivation is inferred from the outcome. The student should hold the claim in this light: it is a powerful and practically useful claim, and it is not a strictly logical one.

The claim of necessity has fared much better. The consistency of the alliance–outcome relationship across therapies, and the research showing that the alliance mediates the delivery of specific technique, together support the proposition that these conditions are necessary for the change that the therapy is trying to produce.

# 5. The Core Therapeutic Conditions in Practice

## 5.1 Empathic understanding

Empathic understanding is the practitioner's accurate grasp of the client's experience, communicated in a way the client can recognise as accurate. It is not agreement, and it is certainly not sympathy. Rogers' formulation of the conditions of empathic understanding — the psychotherapist is in a state of primacy of interest in the client's phenomenological field; the client is the centre of attention; and the therapist attempts to communicate this understanding — is exact about the attitude and imprecise about the skill, and the modern research literature has addressed the skill directly.

The modern finding is significant and clinically useful: **accurate empathy predicts better outcome, and inaccurate empathy predicts worse outcome than a low level of accuracy.** A practitioner who believes they are understanding and is not is doing more harm than one who is careful and tentative. The training literature on practitioner accuracy — the extent to which a practitioner's attunement matches the client's internal state — suggests that this is a learnable skill, and the finding that accuracy improves when the practitioner is given structured feedback is a direct argument for the feedback practices developed in later chapters.

Three clinical failures are common and worth naming. **Intellectualisation** is the substitution of a conceptual account for an emotional response. **Over-identification** is the practitioner's own unprocessed material intruding into the empathic response. **Over-interpretation** is the practitioner's hypothesis offered as the client's experience. All three are more common in the early years of practice than the curriculum's emphasis on technique would suggest.

## 5.2 Unconditional positive regard

Unconditional positive regard is the practitioner's sustained acceptance of the client as a person of worth, without conditions of worthiness, and it is directed at the client as a person rather than at any particular behaviour or statement.

The requirement is demanding in two ways. First, it is demanding of the practitioner's own internal state: it cannot be performed reliably if the practitioner is disapproving, frightened, or bored, and the failure is frequently not a matter of the practitioner failing to try but of the practitioner not having the emotional resources to sustain the stance. Second, it is demanding because the practitioner's genuine disagreement and concern are part of the practice, and the theory's requirement is that the disagreement be held without withdrawal of regard. A practitioner who is being asked to work with a young person who is harming themselves has a genuine concern, and Rogers' framework does not ask the practitioner to abandon it; it asks that the concern not be converted into a withdrawal of acceptance.

The commonest failure is conditional regard in the form of approval and disapproval — the subtle "good" and "not so good" that Rogers and his colleagues described, and that research on the impact of practitioner behaviour on young people suggests can be quite readily detected.

### 5.3 Congruence

Congruence is the practitioner's authenticity — the matching between what the practitioner says and what the practitioner actually experiences, including the awareness and the expression of the practitioner's own feelings, which are in any case the practitioner's real feelings and not the ones the theory might require.

Congruence is the most difficult of the three, and the one most often superficially implemented. A practitioner who says "I can see this is really hard for you" when they are in fact bored has been dishonest; and the dishonesty is not a moral failure but a clinical one, because a client reliably detects incongruence and responds to it by withdrawing. A practitioner who says "I am frustrated, and I notice that I am frustrated, and I want to talk about that" is being congruent, and the invitation is the point.

The value of congruence with young people is considerable, and it is also the condition most likely to alarm a trainee. A young person who is testing — who wants to know whether this adult will survive contact with them — is testing precisely this, and a practitioner whose congruence is a performance of warmth will fail the test and lose the relationship. A practitioner who can be present with their own genuine reaction, including irritation, and who can own it, will usually pass it.

### 5.4 The practitioner's own theory of the person

The fourth condition in Rogers' later formulation is that the practitioner must have a theory of the person, and that this theory must be compatible with the conditions. A practitioner who believes that the client is fundamentally resistant, or that the client is manipulative, or that the client's account is unreliable, cannot sustain unconditional positive regard for long, and the failure will be experienced by the client as a change in the practitioner's stance. The practitioner's beliefs about the client are themselves a therapeutic instrument, and a practitioner who has not examined their own is working with a theory they have not chosen.

## 6. The Therapeutic Process

### 6.1 The client's frame of reference

The organising concept of the client-centred process is the **frame of reference**, which is the internal, subjective, psychological world of the client, including all the perceptions, thoughts, and feelings that are within the client's awareness, together with the client's own view of the situation and of the therapeutic relationship.

The practitioner's task is to enter the client's frame of reference. Rogers described this as the practitioner becoming "a companion in the exploration of the client's world," and the metaphor is exact: the practitioner does not analyse the client's experience from outside, and does not direct the client toward a predetermined destination. The practitioner explores, and the exploration is genuinely mutual — the practitioner is changed by it, and Rogers' claim is that the practitioner is changed in the direction of greater congruence and self-acceptance.

## 6.2 The process of change

The account of the process is the following, and it is a model of a mechanism.

The client is incongruent, and experiences anxiety, defensiveness, and an impaired sense of personal worth, which are the three characteristics Rogers' theory assigns to maladjustment.

The conditions of therapy are present, and the client experiences the practitioner as understanding, accepting, and genuine.

As the client experiences empathy, it becomes possible for the client to communicate the experience that has been denied symbolisation — the experience that is inconsistent with the self-concept.

The communication of the experience into a relationship with a non-defensive, accepting other allows the experience to be fully symbolised, and the self-concept is thereby reorganised to accommodate it.

With the new experience symbolised, the previous adjustment — the distorted behaviour and the anxiety — is no longer required, and the behaviours change. The person is then free to move toward fuller actualisation.

The three characteristics of the therapeutic process are therefore: a new **understanding** of the self, a new **acceptance** of the self, and a new **action** on the self. Rogers' phrase is that the client becomes able to "own" the previously disowned experience, and the three are sequential and causally connected.

## 6.3 The role of the practitioner

The practitioner's function is not to interpret, direct, advise, reassure, or challenge. Rogers' list of what the practitioner should not do is instructive: the practitioner should not argue, interpret, evaluate, or approve or disapprove of the client's expressions, and should not reassure. The practitioner should instead communicate understanding, communicate acceptance, and communicate congruence.

The oft-repeated summary — that the practitioner is a "facilitator" — has misled a generation of students, who have taken it to mean a passive and technique-free stance. The person-centred practitioner is anything but passive. The work of providing accurate empathy under time pressure, in the presence of a young person who is hostile or withdrawn, is among the most demanding things a clinician can do.

# Further Reading for This Chapter

- Rogers, C. R. (1957). The necessary and sufficient conditions of therapeutic personality change. *Journal of Counseling Psychology, 4*(2), 95–107. — Read in full, and read it as a formal argument rather than as a set of principles.
- Rogers, C. R. (1961). *On becoming a person*. Houghton Mifflin. — The primary source for the theory; the first ten chapters are the core.
- Rogers, C. R. (1980). *A way of being*. Houghton Mifflin. — The mature statement, including the reflections on ageing, on marriage, and on the limits of the method with some populations.
- Elliott, R., Watson, J. C., & Rice, C. (Eds.). (2014). *A guide to the person-centred process*. Edward Elgar. — The best contemporary clinical account, and considerably more operationally useful than the primary texts.
- Bozarth, J. D. (Ed.). (1997). *Person-centred theory: Rogerian research*. — For the research base.

# 7. Person-Centred Theory with Children and Adolescents

Person-centred theory was developed with adults, and its application to young people requires more adaptation than is usually acknowledged. Four modifications are necessary, and they are not merely technical.

## 7.1 The developmental modification of the conditions

The core conditions are developmental in their expression, and the literature on the developmental differences in the expression of empathy and of acceptance is directly relevant.

With a younger child, empathy is expressed largely through play, through the quality of attention, and through the reliability of the adult's presence, rather than through verbal reflection. The young child's "frame of reference" is largely pre-verbal, and the practitioner's entry into it is through activity rather than through talk. This is the developmentally appropriate form of the person-centred method, and the tradition's own history contains it in the work of the play therapists who were person-centred in their orientation long before anyone used the phrase.

With an adolescent, the developmental issue is different and more specific. The adolescent's emerging autonomy and the renegotiation of the attachment relationship mean that the adolescent is unusually sensitive to the practitioner's attempts to direct, and unusually attentive to the difference between being treated as a person and being managed. The person-centred conditions are consequently more demanding to maintain and more valuable when maintained, and the adolescent will detect a failure of congruence faster and respond to it more sharply.

## 7.2 The modification of the goal

The goal of person-centred work with an adult is the reduction of incongruence between the organismic experience and the self-concept. With a young person, the goal must be stated in developmental terms, and the developmental task is a more suitable formulation of the destination than the reduction of incongruence. A thirteen-year-old who is in the identity exploration stage is not in a state of incongruence that requires correction; they are doing a developmental task, and the therapeutic task is to make the exploration possible and safe.

A practical formulation that combines the two is that the adolescent's distress is often the consequence of the unsuccessful attempt to make the self-concept and the lived experience congruent, and that the therapeutic task is to support the young person in a fuller, more accurate self-conception that does not require the distortion. This formulation keeps the person-centred core while remaining developmentally appropriate.

### 7.3 The modification of the practical arrangement

Person-centred theory implies a particular kind of session, and the practical arrangements in work with young people often conflict with it. Three examples arise routinely.

**Parental presence.** The developmental literature supports parental involvement in work with young people, and the person-centred tradition supports the young person's autonomy and privacy. The conflict is real and it is not resolved by theory. The practical resolution is to make the arrangement explicit and negotiable with the young person: what the parent is told, what is kept private, and what happens if the young person wants to speak to the practitioner alone. The person-centred principle that the client is the expert applies to the arrangement of the work as much as to its content.

**The frequency and length of sessions.** Thirty-minute weekly sessions with a young person, arranged by a school around a timetable that does not consult the young person, is a structural imposition. A developmental modification would be shorter and more frequent sessions, or sessions whose timing is in the young person's control, and the practitioner should treat the appointment as a matter of the relationship rather than as an administrative detail.

**The setting.** The room in which the session takes place matters, and it is often a matter of institutional convenience. A young person who has spent the day in a corridor outside the counselling office, as Priya in Chapter 5 did, has already had an experience of the setting's relationship to them.

### 7.4 The modification of the "expertise"

The doctrine that the client is the expert is frequently quoted and frequently misapplied. The client is the expert on **their own experience**. They are not the expert on the diagnosis, on the prognosis, on the evidence base, on the risks, or on the options. A young person is the expert on what it is like for them, and this is a real and important expertise, and the practitioner retains an expertise on the process, on the evidence, and on the alternatives. A person-centred practice that surrenders the practitioner's expertise on these matters is not a more respectful practice; it is a less honest one.

The clinical formulation of the point is that the young person is the expert on the meaning of their experience and the practitioner is the expert on the range of ways in which such difficulties are typically maintained and what tends to help — and that the two knowledges are brought together in a collaborative way. A young person who says "I don't want to talk about it" is exercising expertise about their own experience; a practitioner who treats that as a diagnostic datum rather than as a decision to be respected has stopped being a person-centred practitioner and become a symptomologist.

## 8. Modern Developments in Person-Centred Theory

The theory has continued to develop, and the developments are considerable enough that a student should know them.

## 8.1 The person as a process

The work of the 1990s and 2000s, associated principally with the contributions of Kathryn Schubert and of the "person as process" formulations, reframed the theory in ways that resolved some of the tensions in the original. The central claim is that the person is not a thing with a self-concept but a process of self-creation, and that the self-concept is a process rather than a structure. This formulation accommodates the developmental evidence more comfortably, because a process is something that is always becoming, and the theory's emphasis on the person's own direction of movement is preserved.

The clinical consequence is that the practitioner's task is not to restore a previously-integrated state but to support a process of increasing the person's capacity to know themselves. The vocabulary of the theory changes accordingly: the person is "becoming," and the practitioner works with the becoming rather than on the being.

## 8.2 The integration of affect and cognition

The original person-centred theory positioned affect and cognition as largely separate, with the self-concept as the mediating structure. The contemporary integration, associated with the work of Donald Kiesler and of the modern person-centred writers, argues that affect and cognition are not separable in the way the original formulation implied, and that the process of symbolisation is as much a process of affective organisation as of cognitive construction. The clinical consequence is that the person-centred practitioner's attunement to the client's affect is not merely empathic warmth but a technical skill with a specific function, and that the practitioner attending to the discrepancy between what the client says and the affect conveyed is doing something specific rather than merely being supportive.

## 8.3 The social and political dimensions

The most significant contemporary development, and the one with the largest implications for practice with young people, is the re-reading of person-centred theory in terms of power and social conditions. Three propositions anchor this development.

**The power differential is constitutive.** The therapeutic relationship is not a neutral meeting of two people; it is a structured encounter in which one party holds professional authority, institutional power, and, in the case of a young person, a developmental authority that the young person's own culture may not recognise. A theory of the relationship that does not address the power is incomplete.

**The "actualising tendency" is not a universal.** The claim that the person has an innate tendency toward growth is one of the most culturally marked propositions in the professional literature, and it is worth being explicit about its provenance and its function. It functions to justify the practitioner's optimism and the therapeutic frame itself, and it is also one of the propositions that has been most useful in resisting pathologising the young person. The contemporary position is not that the claim is false but that it is a commitment of the practice rather than a finding of the science, and that a practitioner who holds it should hold it consciously and be prepared to examine it.

**Conditions are not sufficient in the presence of injustice.** The person-centred theory's claim that the relational conditions are sufficient for change is most clearly inadequate where the young person's difficulty is produced by material conditions, discrimination, poverty, or danger. A relationship in which the young person is treated with unconditional acceptance will help them, and it will not change the fact that they are unsafe, that they are being racially abused at school, or that the family is without housing. A contemporary

person-centred practice has to hold both, and the honest version is that the relational conditions are necessary and often sufficient for the *therapeutic* work, and are simply insufficient for the practical work, which must be done at the level of the system.

This development converges with the systemic, the social justice, and the ecological perspectives, and a student who reads Rogers in the original and then reads the contemporary critical literature will see the theory's own logic push it in a direction its founder did not intend. Chapter 48 develops this convergence.

# 9. Evidence and Research Support

## 9.1 The alliance

The central empirical claim of the person-centred tradition — that the quality of the therapeutic relationship predicts outcome — is now among the most robust findings in the field, with meta-analytic correlations in the region of 0.25 to 0.30 in adult samples and smaller but consistent effects in adolescent samples. The finding holds across therapies, and it has become the principal evidence for the common factors position discussed in Chapter 6. It is the strongest support for the person-centred theory, and it supports the theory's *relational* propositions more directly than its more specific propositions.

## 9.2 The outcome equivalence finding

The finding that client-centred therapy produces outcomes comparable to other bona fide therapies is the second pillar of the empirical support, and it is the finding that most supports the theory's claim that the conditions rather than the technique are the active ingredients. As Chapter 6 argued, the finding is more limited than it is usually taken to be, and the modern reading is that the conditions are necessary and that specific techniques contribute where they are indicated.

## 9.3 Client-reported outcome

The Client Outcome Questionnaire, developed within the person-centred tradition, operationalises the theory's own criterion of success — the client's own appraisal of progress — and it consistently reveals larger differences between therapies than the standard symptom scales do. The research literature on the CQ is one of the strongest in the field, and it is directly relevant to the argument in Chapter 6 about the systematic bias introduced by reliance on symptom measures.

## 9.4 The fidelity and outcome relationship

The research on the degree to which practitioners actually deliver the person-centred conditions — and the finding that the three conditions, particularly congruence and unconditional positive regard, are the ones most often *not* delivered, and that fidelity to the conditions predicts outcome — is a modern contribution that strengthens the theory considerably. The finding that training in the conditions improves fidelity and that improved fidelity improves outcomes is the empirical case for teaching these skills explicitly rather than assuming them.

## 9.5 The limits of the evidence

Three limits should be noted. The evidence for the person-centred conditions is derived largely from self-report alliance measures that are themselves the most contested part of the measurement enterprise, since the instruments presume a particular account of what a good relationship is. The evidence base for person-centred therapy with children and adolescents specifically is much thinner than for adults, and the developmental adaptations discussed in Section 7 are largely unsupported by controlled trials. And the theory's central proposition about the reduction of incongruence is difficult to operationalise, so the most specific claims of the theory are the least well tested.

## 10. Strengths and Limitations of Person-Centred Theory

**Strengths.** It is the clearest and most complete account of the therapeutic relationship in the field, and it is the account the alliance research has effectively confirmed. It provides an unusually well-specified account of the process of change, and the process is a mechanism rather than a slogan. It is extraordinarily well matched to the developmental and ethical requirements of work with young people, in its respect for the young person as a person and its refusal to treat a young person's behaviour as a defect. It is testable, and it has been tested. And it is the source of most of the field's humane instincts, including the practice of routine outcome feedback, the use of the term "client," and the insistence on the young person's own account.

**Limitations.** It is historically associated with a passive and non-directive style that is a poor fit with the young people who present with the most severe difficulties, and the field's association of the theory with ineffectiveness is not entirely without historical warrant. It offers little specification of technique, which is a genuine problem with a young person whose difficulties are maintained by a contingency and who needs a specific behavioural intervention. Its sufficiency claim is logically vulnerable. And its account of the person, developed in a particular cultural and historical context, has a individualism that the contemporary critical literature has identified as a limitation rather than merely as a fact.

## 11. Cultural and Ethical Considerations

**The cultural location of the theory.** The person-centred theory was developed in the United States in the 1940s and 1950s, and its account of the person is a thoroughly individualistic one: a bounded self, an internal frame of reference, a private world to be explored, a set of needs that are essentially universal, and an actualising tendency that is a property of the individual organism. In cultural contexts in which the self is constituted relationally, in which the "internal" world of a person is substantially a family and community world, in which the appropriate expression of feeling is indirect, and in which the authority of a professional adult over a young person is unquestioned, the theory's core concepts require substantial translation. Chapter 48 develops this.

**The ethical requirement of congruence with power.** The contemporary development that treats the power differential as constitutive of the relationship has a clear ethical consequence. A practitioner who insists on unconditional positive regard for a young person who has been sexually abused by a trusted adult is being asked to sustain a stance that the young person's experience of the world has made impossible, and the honest response is to name the difficulty rather than to perform the stance. Rogers himself described the conditions as the "core" of the relationship and the technical conditions — congruence, unconditional positive regard, empathic understanding — as the "necessary and sufficient" conditions, and the qualification is not merely semantic.

**The ethical requirement of the young person's expertise.** The doctrine that the client is the expert contains a real ethical claim: that the young person's own account of their experience must be treated as a legitimate and necessary input to the work, and that the practitioner who overrides it on the ground that they know better is violating the young person's autonomy in a way that will damage the work. The doctrine also contains a real limit: the young person is not the expert on the diagnosis, the risk, or the options, and a practice that surrenders those is not respectful but negligent. The ethical requirement is to hold both, and to make the holding visible.

**Unconditional positive regard and the duty of care.** The theory's requirement is in tension with the profession's duty to protect, to report, and to act. The tension is real and the theory does not resolve it. The resolution in practice is that unconditional positive regard is directed at the *person*, and that a practitioner can hold a young person's regard while doing what is necessary to protect them — and that the young person can usually be told exactly this, and that telling them is better than performing a regard that has been withdrawn in fact.

## 12. Case Illustration: Maya, Who Did Not Want a Better Mother

**The referral.** Maya, fourteen, is referred by a school wellbeing lead for "low mood and possible self-harm." Maya has been seen for five sessions by a school counsellor, who has noted that Maya presents as "polite, cooperative, quite flat," that she has a "sad face," and that she has mentioned in passing that she "sometimes thinks about not being here." The school wants the referral to be for "assessment of self-harm and family therapy to improve communication."

**The first person-centred session.** The practitioner begins with the identity question rather than with the problem, and the young woman's response is the first substantive data of the case: she says she is "Maya," lists her mother's occupation, her father's job, her brother's year at school, and her own position in the family as "the one who looks after everyone." She does not mention the school, her friends, her hobbies, or anything she likes. The practitioner notices and does not comment, and in the third session asks about it directly: "Is there anything you'd want to be included here? Anyone you'd want me to know about?" Maya says, "I don't really have anything. I'm not a person, really. I'm just the responsible one."

**The exploration.** Over four sessions the picture assembles. Maya's mother works long hours; the father left when Maya was nine; Maya has been the practical carer of her eleven-year-old brother since she was eleven, and has organised her own life around the school run, the meals, and the homework. The mother says that Maya "doesn't complain" and is "a wonderful help." Maya reports that she was sexually touched by an uncle when she was eleven, and told her mother, and that her mother did not believe her and that nothing was done. She has not told anyone since. The "self-harm" referred by the school consists of scratching her forearm when she is angry with her brother, which she has concealed with long sleeves.

**The person-centred formulation.** The self-concept is clear: "I am the one who looks after everyone," "I don't cause trouble," "I am not a person, really." The experiences that cannot be symbolised within that self-concept are her own anger, her own need, her own sexuality, her own injury, and her own account of what was done to her. Each of these, on Rogers' account, must be expressed in a distorted form, and the distortions are observable: the flat affect, the excessive responsibility, the concealment, the self-directed anger in the form of scratching. The anxiety that Rogers locates at the centre of maladjustment is here located in a body that cannot be looked at.

**The intervention.** The person-centred intervention is the provision of the conditions, and it looks almost unbearably simple when described. The practitioner does not confront the mother, does not require the disclosure to be repeated, and does not set the goals. What the practitioner does is attend, with great care, to the three conditions, and to make the acceptance of Maya's experience unmistakable. This is not a passive process. The most demanding single moment in the case is the third session, when Maya says, "You're the first person who hasn't told me I'm brave," and the practitioner feels a strong and complicated reaction — relief, anger on Maya's behalf, a wish to go to the mother immediately, and a recognition that none of these reactions may be acted on in a way that takes the frame of reference away from Maya. Congruence requires the practitioner to say something like: "That isn't what I think about you either. And I'm not going to say you're brave, because I don't think that's what you want to be. I think you want to be allowed to be angry."

That sentence is the therapy. It is also, incidentally, the most accurate thing anyone has said to Maya in three years.

**The developmental and cultural dimensions.** The developmental issue is that Maya is doing a caregiving developmental task that belongs to an adult, and that the task is developmentally incompatible with her own. The cultural issue is that the family's norms of stoicism and of a daughter's role are the source of the self-concept that is producing the distress, and that a culturally responsive practice would examine whether the mother's expectations are a cultural requirement, a personal history, or both — a question the practitioner raises in the family work later, once the relationship can carry it.

**The ethical necessity.** The disclosure of the abuse is present in the material from the third session, and the practitioner must deal with it. The person-centred framework alone would not be sufficient, and the practitioner does not pretend that it is. The practice here is: the disclosure is received in a way that does not require the young person to perform it again; the practitioner says clearly that the abuse was not the young person's fault and that the failure to act on the original disclosure was a failure; and the safeguarding and legal requirements are met in a way that the young person is told about in advance, in plain terms, with the consequences explained. The theory's requirement of unconditional positive regard and the profession's requirement of protection are both honoured, and neither is sacrificed to the other.

**The trajectory.** The outcome in this case is not resolution. It is a young woman who has a part of her own experience available to her, who is in touch with her anger, who has told one other person what happened, and who is engaged with her mother in a way that was previously impossible. The professional judgement is that this is a substantial and appropriate gain, and that the case should not be closed.

## 13. Key Terms

### Key Terms at a Glance

- **Actualising tendency** — The organism's inherent tendency, in Rogers' account, to move toward the fuller development and integration of the person; the foundational premise of the theory.
- **Organismic experience** — The total, ongoing, changing stream of a person's lived experience, comprising the experience of self, of environment, and of the relation between them.
- **Self-concept** — The internal representation of the self, organised, accessible to awareness, and serving as a selection apparatus that governs what the person can learn about themselves.
- **Incongruence** — The discrepancy between the organismic experience and the self-concept; a discrepancy that operates as a filter on access to experience.
- **Real self** — The person's underlying organising experience and potential for fuller functioning, as distinct from the self-concept.
- **Fully functioning person** — A person in close correspondence between organismic experience and self-concept, and therefore in contact with their own experience.
- **Basic psychological needs** — Positive regard, affection, inclusion, and congruence.
- **Unconditional positive regard** — The sustained acceptance of the client as a person of worth, without conditions of worthiness.
- **Conditional positive regard** — The withdrawal or modification of acceptance contingent on the client's behaviour; the commonest failure of the condition in practice.
- **Empathic understanding** — The practitioner's accurate grasp of the client's experience, communicated so that the client can recognise its accuracy.
- **Congruence (of the practitioner)** — The matching of the practitioner's expressed presence with their actual experienced state, including the honest expression of their own feelings.
- **Frame of reference** — The client's internal, subjective, psychological world, including their view of the situation and of the therapeutic relationship.
- **Distortion** — The expression of an experience that cannot be symbolised in a form that is incongruent with the self-concept and consequently maladaptive.
- **Necessary and sufficient conditions** — Rogers' claim that the three core conditions, with the structural conditions, are necessary and sufficient for personality change.
- **Rogers' Q-sort** — A forced-choice instrument for measuring congruence between the self-concept and the organismic experience; the principal operationalisation of the theory's central construct.
- **Client Outcome Questionnaire** — A brief measure of session-level progress in the client's own terms, developed within the person-centred tradition.

**Key Terms at a Glance (continued)**

- **Primal schema** — The organising core of a person's experience in the person-centred account, and the point at which the person as process formulations locate the work.
- **Person as process** — The contemporary formulation in which the self is a process of self-creation rather than a structure, and the person is always becoming.
- **Theoretical and empirical rooting** — The quality of an interpretation that is held in awareness and is genuinely congruent with the client's own experience, distinguished from an interpretation merely plausible.

## 14. Chapter Summary

Client-centred theory emerged in the 1940s from four influences: the Rankian emphasis on the will, Lewinian field theory, the phenomenological tradition, and the psychodynamic practice Rogers left. It inverted every element of the medical model: the practitioner is a facilitator rather than an expert in disorders, the client is the expert on their own experience, the goal is the reduction of incongruence rather than the cure of a symptom, and the practitioner's own congruence becomes part of the theory of the process.

The theory proposes that a person is an experiencing organism with an actualising tendency; that the self-concept is an incomplete and selective representation of that experience; and that maladjustment is a process in which experiences inconsistent with the self-concept are denied symbolisation, are thereby unavailable to awareness, and are expressed in distorted behaviour. The reduction of incongruence — restoring the person's contact with their own experience — is the mechanism of change, and the conditions of growth are the conditions of therapy.

The evidence is strong for the relational propositions and weak for the specific ones. The alliance–outcome relationship is the most replicated finding in the field; the outcome equivalence finding supports the claim about conditions over technique; the client-reported outcome research supports the theory's own criterion of success; and the research on fidelity to the conditions shows that the conditions are delivered inconsistently and that fidelity predicts outcome. The theory's sufficiency claim is logically vulnerable, and its propositions about the person are culturally marked and now the subject of a substantial critical literature.

Application to young people requires four modifications: developmental modification of the expression of the conditions, developmental modification of the goal, practical modification of the session arrangement, and a careful statement of what the doctrine of expertise does and does not license. The case of Maya illustrates all four, and illustrates the point that the conditions are necessary and often sufficient for the therapeutic work while being simply insufficient for the practical work that must be done at the level of the system.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Identify the four influences on Rogers' thinking and give one example of each in the theory.
2. Explain the claim that the self-concept operates as a filter on the person's access to their own experience, and give a clinical example.
3. Describe the process of incongruence in Rogers' account, and explain why the distorted behaviour is "the best available option within the self-concept."
4. State the three core conditions and the three structural conditions, and explain what "necessary" and "sufficient" mean in the 1957 claim.
5. Why is it important that the goal is the reduction of incongruence rather than the installation of a better self-concept?
6. Give a concrete example of accurate empathy, of inaccurate empathy, and of the difference the two make to outcome.
7. Explain why the relationship is the common factor and the technique the specific factor in the person-centred account, using the mediation evidence.
8. What are the four modifications required to apply person-centred theory to a young person, and what evidence supports each?
9. The doctrine that the client is the expert contains an ethical claim and a limit. State both.
10. In the Maya case, what was the role of the practitioner's congruence, and what would have happened if the practitioner had acted on their own anger at the mother?

## 16. Practice Activities

**Practice and Application**

### Activity 10.1 — The Conditions Practicum (2 hours, triads)

Each triad works with a client statement: (a) "I just don't want to be here any more, but I don't know why"; (b) "My parents don't know what to do with me"; (c) "Everyone says I'm fine, I'm just fine." One student plays the client, one the practitioner attempting the three conditions, one the observer using a checklist of the observable markers of each condition — accuracy of empathy, absence of conditional regard, verbalised congruence. The observer's task is to mark the moment at which the condition was lost and the moment at which it was recovered. Rotate.

### Activity 10.2 — The Expertise Boundary (60 minutes, pairs)

Take a young person's statement that expresses expertise about their own experience and a practitioner's knowledge about risk, options, or prognosis. Produce, for each, a sentence that honours the young person's expertise and a sentence that abandons the practitioner's. Then produce a third sentence that does both. The third sentence is the hard one, and it is the one the doctrine requires. The exercise is designed to produce the productive discomfort that the student will need to work through in practice.

### Activity 10.3 — Reading Rogers' 1957 Paper (90 minutes, individual, then seminar)

Read the paper in full, twice: once for the argument, and once as a practitioner, looking for every place at which the argument depends on a premise the reader has not been asked to accept. Bring a list of five such places to the seminar. The paper is a formal argument and should be read as one; a graduate student who has read it carefully will be able to defend it, criticise it, and use it, which are three different achievements.

## 17. Annotated Reading Pathways

**For the theory.** Read Rogers (1961) for the full statement and Rogers (1957) for the formal argument. Read them in that order, because the 1957 paper is much easier to evaluate once the 1961 book has provided the conceptual apparatus.

**For the clinical application.** Read Elliott, Watson, and Rice (2014) for the contemporary operational account. It is considerably more usable with young people than the primary texts, and it is honest about where the original formulations are underspecified.

**For the research.** Read the person-centred research handbook for the empirical base, and then read the contemporary alliance research, which has done more to establish the theory's central proposition than the person-centred literature itself.

**For the critique.** Read the contemporary critical literature on power, on the cultural specificity of the account of the person, and on the limits of relational sufficiency. The theory is better than its critics sometimes suggest, and its critics are right about more than they sometimes acknowledge. A student who can hold both positions will practise better than one who has read only one.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Person-Centered Theory: The Client as Expert** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is rogers' theory of the self, the actualising tendency, and the conditions of therapeutic change. In adolescent work, understanding the model is only the

first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The mechanism to keep in view is how safety, attention, expression, and relational responsiveness allow experience to become available for reflection and choice. The characteristic professional risk is pressuring disclosure, treating emotional intensity as progress, or neglecting risk, family context, and practical action. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in individual sessions where trust, voice, and self-understanding are central, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tobias is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tobias's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tobias explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The first intervention is deliberately modest. Tobias chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tobias has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tobias can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Person-Centered Theory: The Client as Expert

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Person-Centered Theory: The Client as Expert** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Person-Centered Theory: The Client as Expert** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

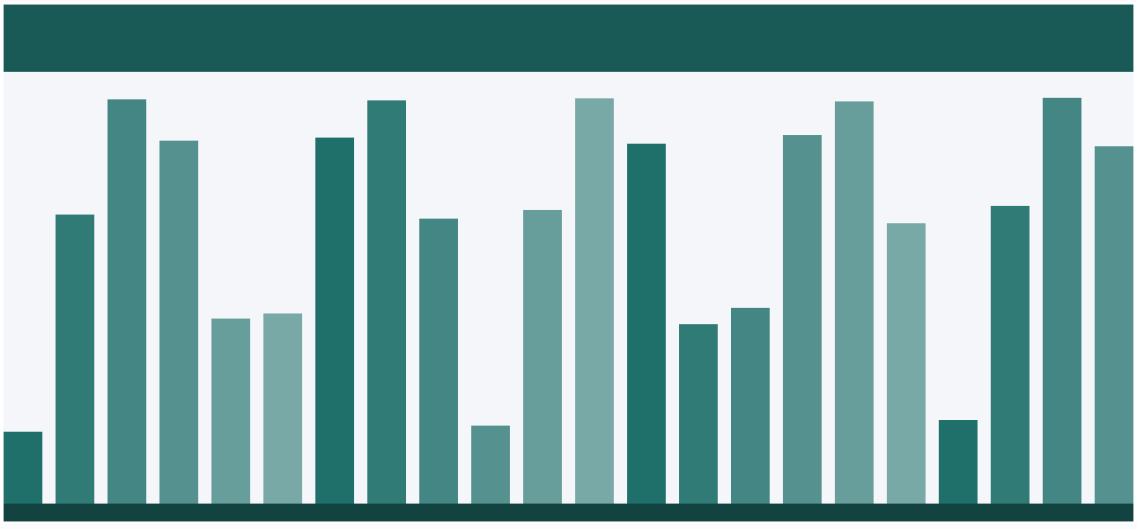
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Person-Centered Theory: The Client as Expert**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Person-Centered Theory: The Client as Expert to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 11

# Existential Therapy: Meaning, Isolation, and Freedom

Yalom, May, Frankl, and the clinical use of existential confrontation.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Trace the intellectual origins of existential therapy in European philosophy and literature, and explain why it entered counseling when it did.
- State the core existential propositions about the human condition and their significance for practice.
- Explain the four givens of human existence — mortality, freedom, isolation, and meaninglessness — and the anxiety each produces.
- Describe the existential account of defence and avoidance, and explain its clinical implications.
- Outline Yalom's four elements of the therapeutic encounter and their developmental adaptation.
- Describe logotherapy's theory of the will to meaning and the techniques it generates.
- Apply existential thinking to work with children and adolescents, and identify the developmental limits of the approach.
- Evaluate the evidence for existential therapy and for the "meaning" constructs on which it rests.

## Chapter Overview

Existential therapy is the only theory in this book that begins with a philosophical account of the human condition rather than with a clinical problem. Its two predecessors, psychoanalysis and the crisis-oriented work of the mid-century European psychiatrists, both treated meaning as something a person might construct to defend against anxiety. Existential therapy inverts the sequence: anxiety is the condition, meaning is the response, and the therapeutic task is to help a person confront the anxiety of the human condition in a way that makes a genuine, chosen commitment possible.

The theory is of particular interest in a course on counseling children and adolescents, and for two reasons that pull in opposite directions. In its favour: adolescence is precisely the period in which the four existential givens become pressing, and the developmental literature on the adolescent identity task describes a young person engaged in exactly the existential questions the theory addresses. Against it: the developmental literature is equally clear that the capacity to hold an existential concern in a reflective, symbolising, non-paranoid way is a developmental achievement, and that an adolescent asked to confront the absurdity of existence at fourteen may be pushed into a psychological crisis that is developmental rather than pathological.

The chapter presents the theory in its intellectual context, develops the four givens and their associated anxieties, describes the defensive structures, presents Yalom's operational formulation of the encounter, covers logotherapy, and then spends considerable space on the developmental question — including the specific adaptations that make existential work with young people possible, and the specific ways in which it can go badly wrong.

## 1. Historical and Intellectual Context

### 1.1 The European roots

Existential therapy draws on three European traditions, and a student who knows only the clinical literature will underappreciate how much of its content is philosophical.

**The phenomenology of Edmund Husserl** supplied the method: the study of the structures of experience as they present themselves to consciousness, and the reduction of the natural attitude in which the world is taken for granted. Existential therapists inherit the practice of bracketing the assumption that the world is as it appears, and the practice of attending to the contents of experience. In the clinical setting, this becomes the practice of listening to the young person's experience as it is, rather than as it is presumed to be.

**The phenomenology of Martin Heidegger** supplied the core ontological claims: that the human being is a being-toward-death, whose distinctive mode of being is an involvement with a world in which the being at stake is the person's own; that the person's fundamental relation to the world is one of care rather than of detached observation; and that the "they" — the impersonal, public, conventional self — is the ordinary mode of being from which the individual self must be won. Heidegger's analysis of the they-self and of authenticity was taken directly into clinical theory by the existential psychiatrists, and it is the source of the clinical concept of the "defences" against anxiety.

**The phenomenology and existentialism of Martin Buber**, with its account of the I-Thou and I-It relations and its account of the self as constituted in relation, supplied the relational dimension that a purely ontological account of the individual lacks.

**The existentialism of Jean-Paul Sartre** supplied the analysis of freedom and responsibility, and with it the sharpest version of the existential conflict: the person is radically free and therefore radically responsible, and the principal means by which people avoid this responsibility is the adoption of a false project, a role, or a self-image that determines the person's course in advance. Sartre's account is the most coherent statement of the existential position and the one that most directly informs the clinical concept of bad faith.

**The work of Kierkegaard, Nietzsche, and the phenomenology of the body in Merleau-Ponty** supplied the account of anxiety as the appearance of the absurd, and of the body as the lived ground of existence.

## 1.2 The clinical introduction

The existential psychotherapies emerged in the Europe of the 1940s, in the context of the Second World War and its aftermath, where the encounter with death, displacement, and the collapse of received meanings was not a philosophical abstraction but a daily reality for both clinicians and patients. Three figures are central.

**Ludwig Binswanger**, whose work on the "being-in-the-world" and on the modes of existence in the *Being and Time* of Heidegger brought existential ontology into the psychiatric clinic, and whose case studies of melancholia, of the "spatiality of the insane," and of the significance of the body remain outstanding as clinical writing.

**Medard Boss**, whose work in daseinsanalysis applied the existential analysis to the practical problems of psychiatric care, and whose emphasis on the "unhiddenness" of being and on the ordinary everydayness of human existence was a corrective to the pathological focus of the psychiatric institution.

**Irvin Yalom**, who brought the European tradition into the American profession in the 1960s, in a form that was immediately usable, that was written in English with clinical precision, and that was demonstrated in a small number of very good books. Yalom's contribution was to make the theory teachable and to give it an operational form in the four elements of the therapeutic encounter.

**Viktor Frankl**, whose logotherapy developed from his work in the concentration camps and which, in its emphasis on the will to meaning, is the branch of existential therapy that has had the most influence outside the profession, including in the education and organisational literatures.

### 1.3 The timing

The reason existential therapy entered the profession when it did is instructive. The 1950s and 1960s saw a profession dominated by the behaviourist and psychodynamic wings, both of which were, from an existential point of view, precisely committed to avoiding the question. The behaviourist commitment to observable, measurable, and controllable behaviour located human difficulty in the realm of the objective, and thereby made the question of meaning invisible. The psychodynamic commitment located difficulty in the realm of the historical, and thereby made the question of the present, of the person's actual situation now, invisible. Existential therapy arrived as the voice of a question that both wings had suppressed, and its subsequent history in the profession has been a history of a theory that has been in continuous tension with the field's commitment to operationalisation.

That tension is the theme of this chapter, and the student should hold it throughout.

## 2. The Core Propositions

Existential therapy rests on four propositions, and a student should be able to state each of them accurately.

**Existence precedes essence.** The person is not a thing with a fixed nature who then acts; the person is a project who acts and thereby becomes something. In the clinical setting, this means that the person is not a set of fixed characteristics to be understood and then modified, but a being in process, whose identity is being constituted in what they do. This is a close cousin of the person-centred account of the self as a process, and the two traditions have been allied since the 1990s.

**The person is free and responsible.** The person is not determined by their history, their genes, their circumstances, or their unconscious; they are free to choose, and therefore responsible for their choices. The clinical significance is enormous and double-edged. It supports the practitioner's respect for the young person's agency and it forbids the attribution of pathology to a person as an explanation. And it is a proposition whose cultural specificity is considerable: the Western philosophical account of the radically free individual is not a universal account of human beings, and Chapter 48 returns to this.

**The person is alone in a fundamental sense.** Existential isolation is not social isolation, which is a circumstance that can be changed. It is the irreducible fact that no one else has access to the centre of one's own experience. It is the condition of the possibility of every relationship, since two people can only be together because they are separately there. Existential therapy's characteristic symptom is a relatedness to the world, in which the person is in profound and unsatisfying contact with the world and with other people.

**Existence is fundamentally meaningless, or at least radically ungrounded.** The world has no inherent purpose, and the person's existence is not guaranteed, is not permanent, and is not part of a larger plan. This does not mean that the world is absurd in a comic sense; it means that meaning is not given and must be made.

# 3. The Four Givens of Human Existence and Their Correspondent Anxieties

Yalom's formulation of the four givens is the standard clinical presentation, and the givens are not four separate topics but four existential facts whose implications are experienced as anxiety.

## 3.1 Mortality

**The existential fact.** The person is finite. Death is certain, the timing is not, and the possibility of dying at any moment is a fact of every moment of life.

**The anxiety.** The anxiety of death is the anxiety of non-being, and it appears in a number of forms: as dread, particularly in the young; as the sense of the meaninglessness of everything in the face of its ending; and, in what Yalom called the "nightmare of the possible," as the possibility that one might never have existed at all, which is a form of anxiety about the arbitrariness of existence rather than about its ending.

**The clinical forms.** Existential therapists observe that the anxiety of death appears in young people as preoccupation with death and dying, as a need to control and to leave a record, as a difficulty with the committing of one's life to anything, and as a preoccupation with the body's mortality. In the counselling room with adolescents, it may appear as an almost theatrical nihilism, as a provocative testing of the adult's reaction, as a preoccupation with the future that has the quality of a refusal to have one, or as a crisis in which the young person has to be talked out of a suicidal plan.

**The developmental qualification.** Suicide in adolescence is, on the developmental evidence, more often a communication and a demand for a change in the situation than an ending of life, and the developmental reading of suicidal behaviour — that the young person is attempting to solve an intolerable situation by the only means available to them — is a much more useful clinical frame than either the existential reading or the diagnostic reading. The existential contribution to it is a single and important insight: that the young person who is in suicidal crisis is frequently in the grip of a coherent system of meaning in which death is the only available exit, and that the work is to enter that system and find another exit, not to argue the young person out of it.

## 3.2 Freedom and responsibility

**The existential fact.** The person is not determined. Whatever the conditions of a life, the person retains the possibility of choosing differently, and is therefore responsible for the choice.

**The anxiety.** The anxiety of freedom is the burden of responsibility, and it appears in three principal forms.

*The anxiety of the possible*: the recognition that a decision has not been made and that not deciding is itself a decision.

*The anxiety of the fallibility of the world*: the recognition that the foundations on which one has relied may not hold, that the authorities on whom one has depended may not be reliable, and that the certainties of one's life may be revealed as contingent.

*The anxiety of the responsibility of the self*: the recognition that one cannot finally be anything other than what one chooses, and that there is no appeal against the sentence of one's own life.

**The clinical forms.** The anxiety of freedom is the most common of the four in clinical practice with young people, and it appears most characteristically as the panic of a life that has become too narrow — the young

person who has been told what to do by parents, by a faith community, by a partner, by a school system, and who experiences the prospect of an open future as intolerable rather than as liberating. It also appears as the anxiety that accompanies a genuine developmental transition, such as leaving home, leaving a relationship, or leaving the last structured setting in which the young person's life has been organised.

**The developmental qualification.** The developmental literature on the adolescent identity task gives a specific and practical form to the existential account of freedom. The adolescent's developmental task is exactly the task of taking responsibility for a set of commitments, and the anxiety of freedom is the anxiety of the identity task. A young person who is panic-stricken about a decision about college, about a relationship, or about a course of study is not being irrational; they are confronting the developmental task in a form that their capacity to bear it does not yet support. The clinical task is developmental: to make the choice smaller, more concrete, more reversible, and more supported, and to accept that the developmental task will be completed across years rather than in a single session.

## 3.3 Isolation

**The existential fact.** No one else has access to one's own experience. There is an irreducible element of aloneness that no relationship removes, and conversely no relationship is possible without it.

**The anxiety.** The anxiety of isolation is the anxiety of the unbridgeable gap, and Yalom described it as the deepest of the four, the one that undercuts every other. It appears in three forms.

*Primary isolation*: the isolation of the infant from the world, before any object or person can be related to.

*Secondary isolation*: the isolation that arises in the course of a life, from the impossibility of fully sharing one's experience with anyone, from the failure of any single relationship to provide a complete answer to the need for relatedness.

*Existential isolation*: the isolation of the individual as such, which is not reducible to the other two and which is a condition of the possibility of the others.

**The clinical forms.** Yalom's observation that the existential therapist encounters the patient's isolation most sharply at the moment of termination — the point at which the possibility of real relatedness has been established and the impossibility of it is simultaneously established — is a clinical truth that every practitioner will recognise, and it is a strong argument for attending to termination early rather than late. The isolation anxiety also appears in young people as a pervasive sense of being fundamentally not-understood, which is frequently mistaken for a cognitive distortion and which is better understood as an accurate description of a condition: a young person with a neurodevelopmental difference, or one who is the only member of their family in a country, or one who is gay in a family that cannot accept it, may have a very accurate sense of being unreachable.

**The developmental qualification.** The developmental literature on the adolescent's renegotiation of attachment is relevant here. The adolescent's isolation is developmental, and the developmental work is the movement from the childhood state of complete inclusion in the family to the adult state of a selective, chosen, and therefore limited set of attachments. A young person who feels isolated is frequently in the middle of that transition, and the work is not to remove the isolation but to enable the young person's making of the choices that will constitute their eventual connection.

## 3.4 Meaninglessness

**The existential fact.** The universe presents no intrinsic purpose, and the person's existence is neither guaranteed nor part of a plan.

**The anxiety.** The anxiety of meaninglessness is the sense of the emptiness, the pointlessness, of a life in which nothing has been chosen because nothing needs to be. Yalom described this as the "hiding in the fog" experience, and the existential therapist's approach to it is neither nihilism nor reassurance but engagement.

**The clinical forms.** Meaninglessness is the most commonly presented existential difficulty in outpatient work with young people, and it appears as a pervasive emptiness, as a sense of going through motions, as the conviction that nothing matters and that this conviction is itself a sign that something is wrong with them, and as a general loss of the capacity to want anything.

**The developmental qualification.** Here the developmental literature is especially useful, because the adolescent's sense of meaninglessness is frequently a developmental stage and frequently a developmental opportunity. The developmental research on the possible self shows that the adolescent's motivation is organised around the future and around the gap between the present self and the possible self, and that a young person who has no future has no possible self, and a young person with no possible self has no motivation. The therapeutic work with a young person who is meaninglessly going through the motions is therefore frequently the work of finding a possible self — of enabling the young person to imagine a future in which something is at stake — and this is a developmental task with a developmental timetable.

## 4. The Existential Account of Defence

The existential account of defence is the most original clinical contribution of the tradition, and it is worth understanding precisely, because it differs from the psychodynamic account in a way that has direct practical consequences.

In the psychodynamic account, defence is a mechanism that protects the person from internal anxiety, and it operates within the personality. In the existential account, defence is a way of **avoiding the anxiety of the human condition** by adopting a **determining system of meaning** that reduces the person's freedom and thereby removes the burden of responsibility.

The structure is: the person encounters one of the four givens; the anxiety is experienced; and the person responds by organising a system of beliefs, values, and commitments that makes the anxiety unnecessary. The "determining system" is the name Yalom gave to the resulting structure, and the account of it is in some ways the most clinically usable part of the existential literature because it is a direct description of what a young person is doing in the counselling room.

A young person who has adopted a determining system of "nothing matters, I am beyond all that" has solved the problem of meaninglessness by deciding that meaning is not available, and has thereby become free of the responsibility of choosing. A young person who has decided that "I am the family's rock" has solved the problem of the anxiety of freedom by making the choice in advance and foreclosing it. A young person who has decided that "I must be completely self-contained" has solved the problem of isolation by pre-empting the need for anyone. A young person who has decided that "I am going to die anyway so it doesn't matter" has solved the problem of mortality by making the ending certain in advance and thereby removing the need to live.

The clinical implication is precise and practical: **the task is not to remove the determining system but to enter it, to understand what it is for, and to enable the young person to relinquish it.** And the

therapeutic stance required is one of acceptance — the practitioner's understanding of the determining system as a solution rather than as a symptom, which is exactly the person-centred stance, and exactly the stance of the therapeutic relationship as described in Chapter 5.

## 5. The Therapeutic Stance and Yalom's Four Elements

Yalom's operational formulation of existential therapy is in four elements, and it is one of the most teachable formulations in the field.

**Presence.** The practitioner is fully available to the young person. Presence is the counterpart of Rogers' congruence, and it is defined in the same register: the practitioner is not offering a technique, and not withholding a reaction, but is genuinely there, with their own experience of the young person's situation available and honestly expressed.

**Empathy.** The practitioner understands the young person's experience from the inside, in a way that is experientially accurate rather than merely intellectually grasped. Yalom is careful to distinguish existential empathy from the empathic understanding of the person-centred tradition, on the grounds that the latter can be satisfied by a practitioner who understands the young person's experience as a set of problems to be solved, while the existential variety requires an understanding of the young person's lived situation, including the young person's relation to their own finitude, freedom, isolation, and possibility.

**Authenticity.** The practitioner is genuine, and the relationship is not a pretense. Authenticity is the strongest and most vulnerable of the four elements, and Yalom discusses at length the ways in which it can fail and the ways in which it can be used manipulatively by a young person — a point that a student should note, because authenticity without structure is a well-documented route to unhelpful therapy.

**Emotional involvement.** The practitioner is emotionally involved, and is willing to be affected by the young person. This is a relatively recent addition to the formulation and reflects Yalom's increasing emphasis on the "therapeutic factor of the encounter."

Yalom also specifies the **prime therapeutic factor**, which is the client's discovery that he or she has a choice, and that there are more choices than had been supposed. The whole of the work, in this account, is the expansion of the young person's sense of the possible.

### 5.1 The developmental adaptation of the four elements

The four elements require developmental modification, and the modifications are as follows.

With a younger child, the work is conducted through play and through the body, and the "givens" are approached indirectly. A seven-year-old's relation to mortality is not a philosophical position but an experience of a grandparent's death, and the therapeutic material is the grief and the questions.

With an adolescent, the four elements are in principle fully available, and the developmental problem is the balance between the adolescent's readiness for the work and the intensity of the anxiety it provokes. The clinical task is to work at the edge of the young person's capacity — the zone of proximal development of Section 3 of Chapter 9 — and to support them in tolerating the anxiety rather than in avoiding it.

## 6. Existential Therapy with Children and Adolescents

Three points of principle, and then a set of practical cautions.

### 6.1 The four givens are developmental facts, not philosophical propositions

A young person does not need to have read Heidegger in order to be confronted with mortality, freedom, isolation, and meaninglessness. A twelve-year-old whose parent has a terminal illness is confronting mortality. A fourteen-year-old who has been told by their family exactly what they will study and do is confronting the question of freedom. A child who is the only one at school who speaks their family's language is confronting isolation. A seventeen-year-old with no future is confronting meaninglessness. The developmental work is to make these givens available as givens, rather than as symptoms.

### 6.2 The danger of premature confrontation

The most serious clinical error in existential work with young people is the premature confrontation of a givens. A fourteen-year-old taken by a practitioner to the question of mortality may experience the encounter as an abandonment, and the young person's response may be a crisis that is developmental in origin and iatrogenic in trigger. The safeguards are: the developmental judgement about readiness; the ordinary pacing of the work; the availability of the practitioner across the crisis; and, where the young person's suicidality is present, the full use of the risk assessment and management procedures that Chapter 54 sets out.

### 6.3 The danger of the philosophical posture

The second danger is that the practitioner adopts the philosophical posture as a performance. Existential language in a young person's hearing can easily read as pretension, and a young person who is told that their difficulty is an "existential dilemma" has been given a vocabulary that is more remote from their experience rather than less. The clinical test is simple and is the same test that Chapter 5 applied to the person-centred conditions: does the young person recognise the description as accurate? If the practitioner is working in language the young person does not use, the work is not in the young person's frame of reference.

### 6.4 The strength of the approach with young people

The strength of the existential approach with young people is that it takes seriously the young person's own questions. A fourteen-year-old who asks, in a session, whether it is pointless to try at anything, is asking an existential question and is usually being answered with reassurance or with a referral. An existential therapist sits with the question, and the sitting is the intervention. The developmental literature supports this: the adolescent's capacity for abstract, symbolic thought and for the consideration of the long-term future develops precisely in this period, and a young person who is met with respect for a genuine question is having an experience that developmental research shows to be rare in the lives of young people, and that promotes the development of exactly the capacities the question exercises.

## 7. Logotherapy and the Will to Meaning

**Viktor Frankl's logotherapy** is the branch of existential therapy with the most influence outside the profession, and it deserves separate treatment because its technique is more specified.

Frankl's theory holds that the primary human motivation is the **will to meaning** — the striving to find and to live a meaningful life — and that this is a more basic motivation than the drives for pleasure (Freud) or for power (Adler). Meaning, in Frankl's account, is discovered through the world and through oneself, and is not manufactured; it is a matter of the person's relation to the reality of the situation.

Frankl distinguished three paths to meaning: by **creating or working** something; by **experiencing** something, particularly love and the natural world; and by **choosing one's attitude** to a situation that cannot be changed. The third path is the one that is most relevant to work with young people in difficult circumstances, and it is the one most often misunderstood as a counsel to endurance.

The logotherapy techniques include **paradoxical intention**, in which the practitioner asks the young person to wish for the thing they fear, on the grounds that the wish transforms the relationship to the fear from a passive to an active one; **existential vacuum**, which names the specific phenomenon of a young person who has lost their sense of the reason for anything and who frequently presents it as a depression; and the **logotherapeutic technique of reflection** in which the practitioner draws attention to the young person's remaining capacities and possibilities.

**Paradoxical intention with young people requires considerable care.** With a young person who is anxious about a school presentation, asking them to hope for the worst is a technique that requires a relational context in which it is experienced as play rather than as cruelty, and with a young person who is depressed, asking them to wish for the thing they most dread can be experienced as a systematic instruction in self-harm. The technique is not a technique to be applied by rule.

## Further Reading for This Chapter

- Yalom, I. R. (1980). *Existential psychotherapy*. Basic Books. — The standard clinical text; the four elements and the four givens are set out in the first half.
- Yalom, I. R. (2008). *The gift of therapy*. HarperCollins. — Yalom's own account of the craft, written with unusual candour, and the best available guide to what he considers the real work.
- Frankl, V. E. (1946). *Man's search for meaning*. Beacon Press. — The primary text, and much more than a case for logotherapy.
- Binswanger, L. (1963). *Being-in-the-world*. Basic Books. — Difficult, rewarding, and the source of the clinical vocabulary.
- Schnegg, A. (2004). *Existential psychotherapy with young people*. — A substantial review of the developmental and clinical literature in this area.
- Buber, M. (1957). *I and Thou*. — A short and life-changing book; read it before reading the clinical literature on relational approaches.

## 8. Evidence and Research Support

The evidence position for existential therapy is more qualified than the theory's standing in the professional imagination, and a student should be able to state the position precisely.

### 8.1 What is well supported

**The clinical relevance of the existential concerns with young people.** The developmental research on the adolescent identity task, on the possible self, on the sense of the future, and on the age-graded development of the capacity for abstract and symbolic thought provides strong support for the clinical observation that the four givens are pressing and developmentally appropriate in the second decade. This is not evidence for existential therapy as a treatment; it is evidence for the relevance of the problems the theory addresses, and it is the most substantial support the theory has.

**The relationship between meaning and adolescent mental health.** The research on meaning in life is substantial and consistent: meaning in life is associated with lower levels of depression, anxiety, and hopelessness, with greater life satisfaction and well-being, with better academic engagement, with greater resilience, and with lower rates of suicidal ideation. The association is bidirectional — the loss of meaning is a feature of depression, and depression corrodes meaning — and the studies are largely correlational, so the causal direction is not settled. The research does establish that meaning is a substantial and modifiable correlate of adolescent mental health, which supports the theory's clinical emphasis.

**The centrality of the therapeutic relationship.** The alliance research discussed in Chapters 5 and 6 supports the elements of the existential formulation that are relational. The finding that the outcome of therapy is predicted by the quality of the relationship, and that this holds across theories including those with no relational component, supports the claim that the conditions of the encounter are the active ingredient.

**The psychotherapeutic and philosophical tradition in the treatment of meaninglessness.** The evidence for the effectiveness of meaning-oriented interventions in a variety of populations, including in the treatment of depression in adults, is moderate and growing, and the "meaning" literature in occupational therapy, in rehabilitation, and in organisational psychology has developed a substantial evidence base of its own.

## 8.2 What is supported with qualifications

**Existential therapy as a treatment.** The outcome studies of existential therapy with adults are small in number and generally of low methodological quality. A small number of controlled trials in the 1980s and 1990s reported favourable results in comparison to wait-list controls and in some cases to other treatments, and the results have never been replicated at a scale that would permit confident conclusions. The systematic reviews consistently rate the evidence for existential therapy as limited.

**Existential therapy with adolescents.** The evidence is thinner still. A small number of controlled studies with young people, most of them in school settings, have reported positive effects, principally on meaning, on hope, and on the reduction of depressive symptoms. The samples are small, the designs are weak, the measures are largely self-report, and the follow-up periods are short. The honest statement is that existential therapy with young people is theoretically well-motivated by the developmental literature, clinically plausible, and empirically under-supported.

**The differential effectiveness of existential therapy.** There is no reliable evidence that existential therapy is more effective than other bona fide treatments for any condition, and the general finding of the field — the equivalence of bona fide therapies discussed in Chapter 6 — applies.

## 8.3 What is weak, contested, or absent

**The theory's metaphysical claims.** The propositions that existence precedes essence, that the person is radically free and responsible, and that existence is meaningless are philosophical claims, and they are not testable and are not supported or refuted by outcome data. A student should be clear that the theory's central propositions are of a different logical type from its clinical propositions, and that the clinical evidence does not bear on them.

**The determinism question.** The apparent contradiction between the existential claim of radical freedom and the psychological literature on the constraints of behaviour, mental illness, and the social determinants of health is not a contradiction within the theory but a tension between the theory and the empirical sciences. The tension has been sharpened by the developmental and neuroscience literature, which locates decision-making in a brain with substantial developmental constraints. A contemporary existential therapist who claims that the young person is entirely free is making a claim that the developmental and neuroscientific evidence does not support, and a student should be able to say so.

**Specificity.** The theory's lack of a specified technique repertoire is a genuine weakness for evidence-based practice, for treatment research, and for a service that needs to specify what it delivers.

**The cross-cultural question.** The existential account of the person is one of the most culturally marked in the field. The claims about individual freedom, individual responsibility, individual meaning, and the individual's relation to death are claims of a Western European philosophical tradition, and the cross-cultural psychology literature finds substantial variation in the endorsed meaning structures, with strong collectivist and relational alternatives. A practitioner working with a young person whose cultural framework locates meaning in relationship, in duty, and in community rather than in personal choice will find that the theory's questions need considerable translation.

## 9. Strengths and Limitations of Existential Therapy

**Strengths.** It takes seriously the questions that young people actually ask, which few therapies do. It locates the difficulty in the person's relation to their life rather than in a disorder, which resists pathologising. It offers a coherent and moving account of the therapeutic encounter, with an operational form in Yalom's four elements. It is a powerful framework for work with young people facing death, terminal illness, severe disability, displacement, and loss, where the meaning questions are the actual clinical questions. It attends to the limits of what a service can do, which is an ethical strength in a field with a tendency to promise more than it can deliver. And it integrates the affective, the cognitive, and the relational in a way that few theories manage.

**Limitations.** It is empirically weak, and its proponents' confidence in it exceeds the evidence by a considerable margin. It has little to offer the practitioner who needs a specific technique for a specific problem, and its generalisation to the full range of clinical presentations with young people is untested. Its language can be grandiose or evasive, and the practitioners who use it are not always the most disciplined about their own. Its cultural specificity is a genuine and serious limitation in a culturally diverse practice. And the question of how to work with a young person in acute crisis — where the existential questions are not accessible because the young person is in a state of terror — is one the theory addresses least well.

## 10. Cultural and Ethical Considerations

**The cultural specificity of the existential individual.** The following claims are all culturally marked and are worth naming individually: that the person is the primary unit of meaning-making; that meaning is a matter of personal choice; that isolation is a universal human condition to be borne; that responsibility for one's existence cannot be shared; and that confronting death is a task for the individual. The cross-cultural literature finds that many communities structure meaning around relationship, duty, filial obligation, and communal identity, and that in many of these contexts the individual's search for personal meaning is experienced as a Western imposition, and that the suffering that is described in existential terms is often experienced as a rupture of a relational or moral order.

The ethical consequence for the practitioner is direct. The existential questions are not neutral invitations; they carry an implicit theory of a good life. A young person from a family for whom filial obligation is the central moral structure may experience the practitioner who asks "what do you want?" as asking them to reject their family, and the "anxiety of freedom" that the practitioner names may be precisely the anxiety of a young person who has been told that their obligations are the source of their confinement. The culturally responsive practice is to hold the question — because the question is real — while holding the family and the culture as a possible answer rather than as an obstacle to it.

**The ethical danger of the existential framing of suicide.** The existential account of suicidal crisis — that the young person is in a coherent system of meaning in which death is the only available exit — is clinically illuminating and ethically dangerous if it is used to interpret rather than to assess. The ethical requirement is that risk assessment and management precede interpretation, that the practitioner does not treat a suicidal young person as being in search of meaning, and that the response to a young person's disclosure of suicidal ideation is immediate, concrete, and grounded in an appraisal of the risk. Chapter 54 sets out the procedures, and Appendix H provides the documentation templates.

**The ethical requirement not to aestheticise suffering.** The existential temptation is to make the young person's suffering beautiful, or significant, or part of a growth process. A young person's depression is not a chapter in a developmental novel, and a practitioner who treats it as one has taken something from the young person and given them a story in its place. The ethical requirement is to take the young person's suffering seriously as suffering, and to be careful with the language of growth.

**The ethical value of the approach.** And the ethical value of the existential approach should also be stated: it is one of the few theories in the field that takes a young person's own existential questions seriously, that declines to answer them with reassurance, and that treats the young person's search as legitimate. For a young person who has been told by every adult in their life that they are too young to think about such things, this is a considerable gift.

## 11. Case Illustration: Theo, Who Did Not Want to Be Helped

**The referral.** Theo, sixteen, is referred by his school for "low mood, disengagement, and a pattern of self-harm." The referral notes that Theo has cut in the past year, has missed substantial amounts of school, and has told a member of staff that "there's no point in any of it." His mother has been contacted and reports that Theo "has always been different" and that she "doesn't know what to do." There is a history of a bereavement: Theo's father died by suicide eighteen months ago.

**The assessment.** The practitioner begins with a risk assessment, because it is required and because the referral is explicit. Theo's self-harm is infrequent, superficial, and typically follows an argument with his

mother. His statement about there being no point is a statement of hopelessness rather than of a plan, and he has no plan, no means, and no history of serious attempt. The risk is assessed as moderate, and a safety plan is made with Theo and his mother, with clear escalation steps. The practitioner also asks the question that the referral does not ask: what did Theo's father's suicide mean to Theo, and what has Theo concluded from it? Theo says, without being pressed: "That you can do it, and that it works. And that everyone ends up doing it eventually, so what's the point of trying to be different."

**The existential formulation.** Theo's difficulty is not primarily depressive in the clinical sense, and the referral's framing is misleading. It is a *determining system* in Yalom's sense, and it is doing serious work. Theo's conclusion — that everyone ends up doing it, so effort is pointless — solves four problems at once. It explains his father's death, so that the death does not require any further meaning. It renders the future unnecessary, so that the anxiety of freedom and the anxiety of meaninglessness are both resolved. It provides a "determining system" in which his disengagement is not a choice but a recognition of the truth, and it gives him a role — the person who has seen it — that carries a certain dignity.

The determining system is also, importantly, a *correct* appraisal in one respect. People do sometimes die by suicide, and Theo is not wrong that the possibility exists. The therapy therefore cannot consist of showing him that the possibility is false, because it is not.

**The first intervention.** The practitioner's first move is not an interpretation. It is a question about the specific: "You said everyone's ends up doing it eventually. Can I ask you about your father? What was he like?" This is the existential move — entering the determining system rather than arguing with it — and it is also the developmentally appropriate move with a sixteen-year-old, who is capable of a real dialogue about his father and who will not tolerate being psychoanalysed.

**The work.** Over the following sessions, Theo talks about his father, and the practitioner follows him into the territory without steering. What emerges is that Theo was the one who found his father, and that he has organised his entire life since around a single, unspeakable guilt: that he could have done something. The determining system is not a philosophical position. It is a catastrophic scene in which every effort is futile because he has already failed once in the most important matter of his life.

The therapeutic work is then straightforward and difficult. The practitioner works to establish that what happened that evening was not within Theo's power to alter, which requires the practitioner to spend a great deal of time on the specific and concrete circumstances of that evening, in the manner of a trauma-focused approach, and to help Theo to construct a version of the evening in which his presence or absence makes no difference, which is true and which Theo can verify. The existential framing is present throughout: the work is to enable Theo to relinquish a determining system that has cost him his future, and to make a different set of commitments available. But the work is cognitive, in the sense that it involves a specific reorganisation of a specific belief, and it is behavioural, in the sense that it involves the graded re-engagement with school and with his friends.

**The developmental dimension.** Theo's case is a developmental case as well as an existential one. The developmental literature is clear that the death of a parent in adolescence is one of the most significant developmental disruptions there is, and that it produces predictable disruption of the identity task, of the attachment system, of the peer relationships, and of the future orientation. Theo has responded to this disruption by an arrest of the developmental process, and the therapy is in part a resumption of the developmental process, and the framework in Chapter 9 is as useful here as the existential one.

**The ethical dimension.** Three ethical requirements were engaged. The first is the requirement that the risk be assessed and managed before the existential work begins, and the practitioner notes in the record that the order was deliberate. The second is the requirement not to aestheticise Theo's father's suicide, which is a live risk in this case, because the account of a father's suicide in a family narrative is very often a story in which the death was a completion of a meaningful arc, and the practitioner repeatedly returns to the specific and the ordinary: what the father was like, what he did on ordinary days, what was said in the car. The third is the requirement to include Theo's mother appropriately. She is, on the evidence, one of the two people most damaged by the death, and her account of Theo as "always different" is a deflection from her own grief. The work with the two of them is the conjoint part of the therapy, and the practitioner is careful to frame the work with Theo as a way of helping him, rather than as a way of helping her.

**The outcome.** Theo's improvement is partial and real. The self-harm stops. The school attendance improves substantially. The determining system loosens: Theo says, in the twelfth session, that he has begun to think about doing something after he leaves school, and that this is new, and that it is frightening. The therapy continues at a reduced frequency, and the termination is planned around the end of the school year, with the themes of the future developed as the explicit agenda. The practitioner judges the outcome to be good but not complete, and records the residual risk explicitly, which is the appropriate thing to do.

## 12. Key Terms

### Key Terms at a Glance

- **Existence precedes essence** — The Sartrean proposition that the person is a project who becomes something through choosing and acting, rather than a fixed nature who then acts.
- **The givens of existence** — The four existential facts — mortality, freedom and responsibility, isolation, and meaninglessness — whose implications are experienced as anxiety.
- **The nightmare of the possible** — The anxiety arising from the recognition that one might never have existed; the arbitrariness of existence rather than its ending.
- **Existential anxiety** — The anxiety that arises from the confrontation with the givens of existence, as distinct from the anxiety of a threatening situation.
- **The they-self** — The impersonal, conventional mode of being in which the person takes over the general view and thereby avoids the anxiety of individual existence.
- **Determining system** — Yalom's term for the structure of beliefs and commitments adopted to avoid the anxiety of a given, which thereby forecloses choice and takes responsibility off the person.
- **Bad faith** — Sartre's term for the flight from the anxiety of freedom through self-deception, particularly in the determination of one's own identity by a fixed project.
- **Existential vacuum** — The specific emptiness of a life in which nothing has been chosen because nothing needs to be; Frankl's term for the phenomenon most often presented as depression.
- **Existential isolation** — The irreducible condition of no one having access to one's own experience, which underpins every relationship.
- **Primary, secondary, and existential isolation** — Yalom's threefold distinction, distinguishing the infant's isolation, the isolation arising in a life, and the isolation of the individual as such.
- **Prime therapeutic factor** — Yalom's term for the client's discovery that there are more choices available than had been supposed.
- **The four elements** — Presence, empathy, authenticity, and emotional involvement; Yalom's operational formulation of the existential therapeutic stance.
- **Existential empathy** — An understanding of the client's lived situation, including their relation to finitude, freedom, isolation, and possibility, rather than an understanding of their experience as a set of problems.
- **Will to meaning** — Frankl's proposition that the primary human motivation is the striving to find and live a meaningful life, which is more basic than the pursuit of pleasure or power.
- **Paradoxical intention** — The logotherapeutic technique of asking the client to wish for the thing they fear, in order to transform a passive relationship to the fear into an active one.

**Key Terms at a Glance (continued)**

- **Logotherapy** — Frankl's theory and method, organised around the will to meaning and the three paths to meaning.
- **Responsible choice** — The clinical requirement that the young person be helped to make and carry out a choice, rather than being relieved of the burden of one.
- **Unfinished business** — The clinical phrase for the material that a young person brings to the session that concerns their dead, and which the existential practitioner treats as legitimate material rather than as a symptom.

## 13. Chapter Summary

Existential therapy is the only theory in this book that begins with a philosophy of the human condition. It derives from Husserl's phenomenology, Heidegger's ontology of being-toward-death and the they-self, Buber's relational account of the self, and Sartre's analysis of freedom and responsibility, and it entered the profession through Binswanger, Boss, Yalom, and Frankl.

Its four propositions are that existence precedes essence, that the person is free and responsible, that the person is irreducibly alone, and that existence is ungrounded. Each of the four givens produces a characteristic anxiety — of mortality, of freedom and responsibility, of isolation, and of meaninglessness — and the clinical contribution of the theory is the account of defence as the adoption of a determining system: a structure of meaning that eliminates the anxiety of the givens by taking responsibility away.

Yalom's four elements — presence, empathy, authenticity, and emotional involvement — make the theory teachable, and the developmental modifications of these elements make it usable with young people. The developmental literature provides strong support for the *relevance* of the existential concerns in the second decade, and the meaning research provides substantial evidence for the association between meaning and adolescent mental health, while the evidence for existential therapy as a specific treatment remains limited.

The approach's greatest strength with young people is that it takes their existential questions seriously, and its greatest dangers are the premature confrontation of a given, the philosophical posture performed as technique, the aestheticisation of suffering, and the cultural specificity of the account of the person it takes as universal. The case of Theo illustrates the theory's power and its risks, and shows that the framework must be combined with a risk assessment, with developmental perspective, and with specific cognitive and behavioural work.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Trace the four European philosophical sources of existential therapy and give one clinical idea from each.
2. State the four propositions of the theory and explain why each is a philosophical rather than a clinical claim.
3. Describe the three forms of the anxiety of freedom, and give an example of each with a young person.
4. Explain the concept of the determining system, and use it to account for a young person's disengagement from school.
5. Compare the existential account of defence with the psychodynamic account, and state the practical consequence of the difference.
6. Define Yalom's four elements, and state the risk that accompanies each.
7. What is the prime therapeutic factor, and why is it difficult to demonstrate empirically?
8. Identify the three dangers of existential work with young people, and state the safeguard for each.
9. In what respects is the theory's account of the person culturally marked, and what is the ethical consequence for a practitioner?
10. In the Theo case, why was the risk assessment done first, and what would have happened without it?

## 15. Practice Activities

**Practice and Application**

### Activity 11.1 — The Givens with a Young Person (2 hours, groups of four)

Each group takes one of the four givens and works out three developmentally appropriate ways of entering the conversation with a young person of a specified age: one who would recognise the question, one who would engage with it obliquely, and one for whom it is premature. The groups then consider what the practitioner would say if the young person responded with a shrug, with irritation, and with a flood. The exercise builds the practical skill and it also exposes the difficulty, which is the point.

### Activity 11.2 — The Determining System Audit (90 minutes, individual, then pairs)

Take a young person you know — a client, a placement student, a family member, a friend — and identify the determining system in operation. Write it as a set of two or three beliefs, in the young person's own words if you can, that make a decision unnecessary. Then write the anxiety that the system is avoiding. Then write what it would cost the young person to relinquish it. The exercise is uncomfortable and revealing, and it develops the observation skill that the theory requires.

### Activity 11.3 — The Logotherapy Technique Under Scrutiny (60 minutes, groups of three)

Each group takes one logotherapy technique — paradoxical intention, the reflection technique, or the logotherapy interview — and constructs a scenario with a young person in which the technique would be helpful, one in which it would be neutral, and one in which it would be harmful. The third scenario is the requirement, and the exercise is designed to produce the caution that the technique requires.

## 16. Annotated Reading Pathways

**For the philosophy.** Read Buber (1957) first, because it is short and because it will make the clinical material intelligible. Then read the clinical accounts, and return to Heidegger and Sartre afterwards, if at all, with the clinical vocabulary in place.

**For the clinical application.** Read Yalom (1980) for the theory and the four elements, and Yalom (2008) for the craft. Yalom's second book is the one to read first, despite being the later, because it is the more honest about what the work actually consists of.

**For the developmental and empirical material.** Read the contemporary literature on existential therapy with young people, and then read the meaning-in-life research literature, which is substantial and considerably better evidenced than the therapy literature. Note the difference in evidential quality, and consider what follows from it for a service that must specify what it offers.

**For the critical reading.** Read the cross-cultural existential psychology literature, and the philosophical critiques of the free will proposition in the light of neuroscience. A student who has read only the clinical accounts will have a picture of the theory that its own literature would not support.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Existential Therapy: Meaning, Isolation, and Freedom** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is yalom, May, Frankl, and the clinical use of existential confrontation. In adolescent work, understanding the model is only the first step. The

practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The mechanism to keep in view is how safety, attention, expression, and relational responsiveness allow experience to become available for reflection and choice. The characteristic professional risk is pressuring disclosure, treating emotional intensity as progress, or neglecting risk, family context, and practical action. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in individual sessions where trust, voice, and self-understanding are central, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Ines is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Ines's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Ines explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The first intervention is deliberately modest. Ines chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Ines has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Ines can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Existential Therapy: Meaning, Isolation, and Freedom

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Existential Therapy: Meaning, Isolation, and Freedom** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Existential Therapy: Meaning, Isolation, and Freedom** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

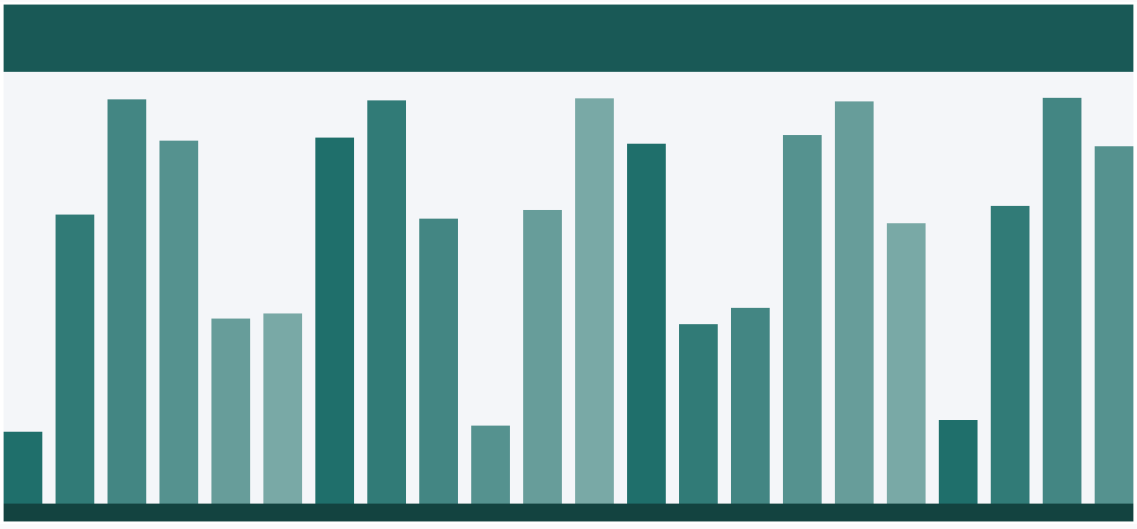
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Existential Therapy: Meaning, Isolation, and Freedom**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Existential Therapy: Meaning, Isolation, and Freedom to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 12

# Gestalt Therapy: Awareness, Contact, and the Here and Now

Perls, Foulkes, and the experimental phenomenology of the counseling session.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Trace the origins of Gestalt therapy in the perceptual psychology of the Berlin School and the phenomenology of the Würzburg school, and describe the influence of Fritz Perls' own clinical and wartime experience on the theory.
- State the core propositions of Gestalt theory and explain the significance of the organism–environment field.
- Explain the three zones of awareness and their clinical use.
- Describe the cycle of experience, the concept of the interrupted gestalt, and the therapeutic significance of the concept of the "unfinished business".
- Explain the meaning of contact and the here-and-now principle, and apply them to a case.
- Describe the principal techniques — the empty chair, the hot seat, the gestalt prayer, and the dream work — and state the indications and cautions for each.
- Apply Gestalt theory to work with children and adolescents, and identify the developmental modifications required.
- Evaluate the evidence for Gestalt therapy and for the "experiential" methods generally.

## Chapter Overview

Gestalt therapy is the most direct clinical descendant of the perceptual psychology of the Berlin School, and the most misunderstood. The popular image — the empty chair, the shout, the encounter with the here and now — captures the techniques and omits the theory, and the omission matters, because the techniques of Gestalt therapy are derived from a specific and rather precise account of how a person functions, and they are much less useful when detached from it.

The theory is built on three propositions from the experimental psychology of the whole. The first is that perception is organised into wholes, and that the mind does not receive elements and assemble them but responds to organised configurations. The second is that a person is not a collection of parts but a dynamic field in which the person and the environment are mutually determining. The third is that the organising principle of the field is a tendency toward a coherent, completed, and differentiated whole — the actualisation of which is what the word *Gestalt* was borrowed to name.

The clinical implications follow with a directness that is unusual in the field. If a person is a field organised toward completion, then a symptom is an uncompleted gestalt, and the therapeutic task is to complete it. If a person is only knowable in the here and now of the relationship, then the most informative data available to the practitioner is the present moment. And if the person and the environment are mutually determining, then the practitioner's own presence is a constitutive part of the field, which makes the technique of confrontation — the deliberate disruption of the young person's habitual way of being — a legitimate and powerful intervention.

This chapter presents the theory in its intellectual context, works through its central constructs, describes the technique repertoire with its indications and cautions, and addresses the developmental and ethical questions.

# 1. Historical and Intellectual Context

## 1.1 The Berlin School and the laws of organisation

The experimental psychology of the Berlin School in the early twentieth century produced a series of findings that Gestalt therapy took as its foundation, and a student of the theory should know them, because the clinical constructs are direct translations of the perceptual findings.

**The law of Prägnanz.** Every stimulus pattern is perceived in the most simple, stable, and symmetrical form possible. The law states that the perceptual organisation tends toward the simplest, most stable, and most complete whole that the stimulus field permits.

**The law of good continuation.** Elements arranged in a line or curve are perceived as related, and the organisation tends to continue smoothly rather than to break.

**The law of closure.** The mind tends to fill in gaps and to perceive incomplete figures as complete.

**The law of figure and ground.** Every field is organised into a figure, which stands out and is attended to, and a ground, which recedes. The figure is the current focus of attention and of organisation, and the ground is what recedes from the foreground but remains in the field.

**The phi phenomenon and the apparent motion.** A sequence of still images perceived as motion; a demonstration that the whole is not the sum of its parts.

The clinical translation of the law of Prägnanz is the theory's central claim: there is a tendency in the organism toward an organised, coherent, and completed whole, and this tendency is the ground of growth. The clinical translation of figure and ground is the account of attention and of the way in which a person organises their experience: what is in the foreground, what has been pushed to the background, and what has been excluded from the field altogether.

## 1.2 The Würzburg school and the phenomenology of the act

The Würzburg school, working at the turn of the century on the phenomenology of thinking, established that thought involves an attitude of mind directed toward an object, that the object of thought determines the experience of thinking, and that there are properties of the act of thinking that are not reducible to its content. The contribution of the Würzburg school to Gestalt therapy is the notion that a person's experience is always experience *of* something, directed toward something, and that the direction of the attention is a clue to the state of the person.

## 1.3 Kurt Goldstein and the organism

Kurt Goldstein's work in the clinical neurology of the 1920s and 1930s, and his concept of the *Gestalt* as the form of the activity of an organism, supplied the biological grounding of the theory. Goldstein's studies of patients with brain injuries established that the organism, when it cannot complete an action, reorganises its whole activity around the incomplete action — and that the incomplete action persists as a drive that organises the total behaviour. This is the physiological counterpart of the clinical concept of the unfinished gestalt, and it is one of the reasons the theory has a certain explanatory power that its reputation for eccentricity sometimes obscures.

## 1.4 Perls: the biography and its consequences

Fritz Perls' path to Gestalt therapy was unusual and it shaped the theory in identifiable ways.

He trained in Vienna in the psychoanalysis of Freud and in the ego psychology of Anna Freud, and in 1930s Berlin he was a student at the Berlin Psychoanalytic Institute, where he was analysed by Kurt Goldstein and worked in Goldstein's neurological clinic. He moved to the United States in the 1930s, and there he encountered the behaviourism of the era as a challenge to the psychoanalytic method — and, in the "empty chair" experiment at the end of the 1940s, at a 1942 military hospital for what was then called "shell shock", he conducted group sessions in which he had one soldier sit in an empty chair and speak to the persona he most disliked.

The Second World War's contribution to the theory is direct and important, and it is the part of Gestalt therapy that most distinguishes it from the other humanistic theories. Perls' work at the military hospital produced the observation that many of the difficulties presented by the soldiers were not difficulties of feeling but difficulties of *contact* — that the person was cut off from the here and now, was living in a dissociated state, and that the re-establishment of contact with the immediate present moment was the treatment. The theory's emphasis on the here and now is therefore not a philosophical position; it is a clinical observation from a specific and very large sample of acutely disturbed people.

The New York school of the 1950s, in which Perls developed the theory with colleagues including Virginia Satir, Laura Perl, and Isadore From, produced the version of Gestalt therapy that is now most familiar, with the emphasis on the techniques and on confrontation. The Chicago school, and the Esalen and humanistic movements of the 1960s, produced a different version, with a much heavier emphasis on the "authentic" and "warm" dimensions and a corresponding softening of the confrontational techniques. The difference matters clinically, and a student should know which version they are reading.

## 2. The Core Propositions

### 2.1 The organism–environment field

The person is not an entity and the environment is not a separate entity; the person is a **field** in which the person and the environment are mutually determining. The word "field" is used in a specific sense: a bounded region of activity in which the parts are determined by their relations with each other and with the whole.

Three clinical consequences follow immediately.

**The practitioner's presence is constitutive.** The practitioner is a part of the young person's field. Whatever the practitioner does — asks, waits, challenges, stays silent — is a contribution to the field and is a possible stimulus for the young person's organisation. This is the ontological justification for the here-and-now principle and for the technique of confrontation.

**The symptom is a function of the field.** A symptom is not located in the person; it is the person's current mode of organising the field, and it arises out of a difficulty in the person's relation to the environment. A young person who is always the responsible one has found a way of organising the field in which they are not responsible for anything, and the symptom is the solution.

**The boundary is negotiable.** The boundary between the person and the environment is not fixed; it is drawn by the person, continuously, in the light of their needs. Symptoms are the drawing of a boundary that is too tight — an over-inclusion of what is not the person's own, an identification with the environment —

or too loose. The account of identification and projection as disturbances of the boundary is one of the theory's most distinctive clinical concepts.

### 2.2 Here and now

The **here and now** is the total situation of the present moment in the field, including the practitioner. The here-and-now principle states that the most significant data available to the practitioner is the present moment of the encounter, and that the work is to bring into awareness what is occurring now.

The principle is more subtle than the popular version, and the subtlety is important. It does not mean that the past is unimportant — the past is present in the field, in the person's posture, in the tone of voice, in the way they arrange the chairs. It means that the past is available for examination only in the present, and that the practitioner's attention to the here and now is the most direct route to the material.

The clinical formulation of this is the **experiment**: the practitioner deliberately alters the field — in tone, in distance, in timing, in warmth, in challenge — in order to create, in the young person, an unfamiliar and disorganising experience, and then observes what the young person does with it. The therapeutic effect of the experiment lies in the difference between the young person's habitual mode of organising the field and their response to the disruption.

### 2.3 Awareness and the three zones

The state of **awareness** is a continuum with three zones, and the zones are used in Gestalt work to locate where a person is attending.

**The zone of awareness** contains what the person is aware of in the present moment, including their sensations, their feelings, their thoughts, their actions, and their interactions with the practitioner.

**The zone of pre-awareness** contains material that is accessible but not currently attended to — the background hum of the session, the physical tension in the shoulders, the half-formed thought, the thing the young person has been circling.

**The zone of unaware material** contains material that is not available to awareness at all, and which is held out of the field by the very process that manages the disruption. It is the material that the person's characteristic mode of organising the field excludes.

The clinical work is the expansion of the zone of awareness, and the technique for it is the **excitation**: the deliberate slight disruption of the field that brings the pre-aware material into focus. The precondition is that the young person has sufficient contact with the field to register the disruption at all, and the state of the person's contact is therefore the first thing to be assessed.

## 3. The Cycle of Experience and the Interrupted Gestalt

### 3.1 The cycle

The theory proposes a **cycle of experience** in which the organism moves through a sequence of phases: an approach toward a need or a contact situation; a heightened awareness at the point of contact; a full, satisfying, and complete contact; the integration of the new experience into the organism's structure; and a withdrawal to the organism's new equilibrium, in preparation for the next approach.

The theory's claim is that this cycle is completed many times a day in ordinary life, that the completion of each gestalt is what gives life its sense of movement and vitality, and that the organism's growth is the progressive capacity to complete more of these cycles and to complete them with greater differentiation.

### 3.2 The interrupted gestalt

The clinical account of difficulty follows. When the organism approaches a situation and the contact is *interrupted* — by a fear, by an environmental obstruction, by an internal prohibition — the organism cannot complete the gestalt, and the incomplete gestalt persists as an **unfinished business**, a drive that organises the person's behaviour around the incomplete contact.

The consequences of the incomplete gestalt are three.

**The figure persists.** The incomplete gestalt remains in the foreground of the person's attention, and the person continues to orient their behaviour towards completing it. A young person who has been unable to say goodbye to a parent who left will find the goodbye in every available relationship, and will organise their life around the attempt to complete it.

**The contact is interrupted by a substitute.** The incomplete energy is channelled into a substitute contact that cannot complete the gestalt either, and the behaviour that results is the symptom. A young person who was unable to assert and was therefore not met with an assertion has substituted an activity that accomplishes the same thing in a form that can be tolerated — and the substitution becomes the pattern.

**The energy is split.** The incomplete energy divides between the pursuit of the unattainable gestalt and the management of the rest of the field, and this is the source of the divided or split quality that is often characteristic of a young person's presentation. The therapy's aim is to allow the energy to be released and the cycle to be completed.

### 3.3 The therapeutic task

The therapeutic task is therefore: to help the young person become aware of the unfinished business, to bring it into the here and now, to allow the feeling to be felt to its completion, and to integrate the completed experience into the person's structure. The theory's language is that the practitioner helps the young person to "close" the gestalt, and the classical figures in the work are the Polar Bear and the Empty Chair: the completion of an unexpressed feeling toward a person who is not present.

The most important clinical point about this account is that it makes the symptom intelligible as an **attempted solution** rather than as a failure. The young person who is always the responsible one is not failing to be a child; they are completing a gestalt that could not be completed directly. The work is to find out what the attempted solution is for.

## 4. Contact and Its Interruptions

**Contact** is the boundary event in which the organism and the environment act upon one another. Good contact is characterised by three conditions: the **presentment**, in which the person is present in the moment and available to the interaction; the **focusing**, in which the person is aware of and engaged with a specific aspect of the field, the figure; and the **full contact**, in which the person is aware of the figure *and* of their own part in it.

Interruptions of contact take several forms, and a student should be able to name them in a case.

**Premature closure.** The person completes a gestalt before the contact is complete — a young person who agrees enthusiastically to everything, who ends the session before the work is done, who talks continuously to avoid the pause in which something might be felt. Premature closure is a form of the "should" of the character structure, and it is a very common presentation with young people who have been over-protected.

**Retroflection.** The person turns back on themselves a communication that should be directed outward. A young person who cannot say "I'm angry with you" and instead becomes depressed, apologetic, or anxious has retroflexed. Retroflexion is the most common form of contact interruption in clinical work with young people.

**Projection.** The person attributes to the environment what belongs to the self, and the attributed material is then perceived in the environment. A young person who insists that everyone in the household is out to get them may be experiencing their own exclusion.

**Confluence.** The person dissolves the boundary with the environment in order to avoid the anxiety of the boundary. The confluent young person is the one who merges with the practitioner, agrees with everything, and is in effect not present in the session at all. The theory holds that the movement from confluence toward differentiation is a central developmental task, and this claim has an obvious application to work with young people whose families have not permitted differentiation.

**Deflection.** The person turns away from the field — into abstraction, into questioning, into an activity, or out of the room. Deflection is the most benign of the interruptions and the one most easily mistaken for engagement.

# 5. The Technique Repertoire

## 5.1 The empty chair

The empty chair technique places an empty chair in the field, and the young person is invited to sit in it and speak to the chair as though it were occupied by the person to whom they have something unsaid. The Polar Bear variation involves the young person making the noise of the animal. The technique is derived directly from the theory: the empty chair represents the absent element of the field, and speaking to it completes the interrupted contact that could not be completed with the real person.

**Indications.** The technique is indicated where there is clearly unfinished business with an absent person, where the young person has difficulty expressing a feeling directly, where the feeling is one of the "unfinished business" type, and where the relationship to the absent person is not dangerous.

**Cautions, which are substantial.** The technique should not be used where the absent person is a source of danger, where the young person has a history of dissociation or of trauma that is not adequately prepared, or where the young person is likely to be overwhelmed. The technique can be experienced as a manoeuvre by the practitioner, particularly in its more dramatic forms, and a young person who is suspicious of adults will sometimes experience the invitation to speak to a chair as a trick. And the technique can put the young person in contact with very large feelings without the containing structure that they need, and the practitioner must be able to stay present with the flood and to help the young person complete it rather than to shut it down.

## 5.2 The hot seat

The hot seat places the young person in the position of the practitioner and the practitioner in the position of the young person, and the young person is invited to treat the practitioner as they have treated others. The technique is an experiment in the strict sense: it disrupts the young person's habitual position in the field and makes available the experience of being on the other side of the interaction.

**Indications and cautions.** It is indicated with young people whose habitual mode is highly compliant, passive, or deferential, because it gives them a position from which to act. It is contraindicated with young people who are fragile, who have been shamed, or who will experience the technique as a replication of a coercive encounter. The practitioner must be prepared to be attacked, and the preparation is not intellectual but attitudinal.

## 5.3 The gestalt prayer

The gestalt prayer is a personalisation exercise: the young person is asked to phrase a statement of a feeling in the first person singular, addressed to the person to whom it belongs — "I am angry with my father and I want him to know." The theory's account of the phenomenon is that the pronoun "I" is used in place of a more distal one because the person is identified with the feeling and with the projected material, and the shift to the first person is the de-identification that allows the feeling to be owned.

## 5.4 The dream work

In dream work, the young person reports a dream, and the practitioner helps them to construct a first-person narrative of the dream, working with each element in turn: what is the place like, what is the texture, what do the figures want, what do they do, what do you feel. The theory's account is that the dream is a gestalt in the process of completion, that every element is part of the young person's own psyche, and that the work of the session is the completion of the dream's unfinished action.

**Caution.** Dream work is contraindicated in young people with a history of dissociation or of nightmares, and with young people in acute crisis, and it is contraindicated in any situation in which the practitioner lacks the training to hold a young person's material without structuring it. It is also the technique most likely to be misused by an inexperienced practitioner, and the literature contains a number of documented cases of harm.

## 5.5 The confrontation of the here and now

The core intervention, and the one with the widest application, is the practitioner's direct statement of what is happening in the field. "Your shoulders have come up and your voice has got smaller since I asked about your father." "You have been answering my questions with questions." "You moved the chair away from me and I noticed you did it."

The intervention is powerful and it is the one that most often frightens students, because it feels like a lack of kindness. The theoretical defence is that the intervention is a gift: it gives the young person information about themselves that they could not otherwise obtain, and it does so in a form that the young person can verify. The clinical caution is that the intervention must be timed to the young person's contact with the field, and a practitioner who confronts a young person who is not in contact is offering an interpretation without the ground to receive it.

# 6. Gestalt Therapy with Children and Adolescents

## 6.1 The developmental fit

The developmental fit of Gestalt therapy with young people is good, and better than it is usually claimed to be. Three features support it.

**The emphasis on the body is developmentally appropriate.** The theory's attention to posture, to gesture, to the sound of the voice, to the placement of the body in the room, and to the felt sense is congruent with the developmental fact that a young person's emotional state is expressed in and organised through the body, and with the somatic and trauma-informed literatures developed in Part VI.

**The emphasis on the present moment matches the developmental condition of the child.** A young child's past is in the present, and a child's work is in the present, and the here-and-now principle is a close fit with a way of working in which the past is reached through what is happening now.

**The focus on the boundary and differentiation matches the developmental task.** The developmental literature on the second decade describes the adolescent's task as the achievement of a differentiated position within a continuing relationship with the family, and the Gestalt account of confluence and differentiation describes the same work in a way that is directly usable.

## 6.2 The developmental modifications

**With younger children**, the work is conducted through play and through the body, and the "unfinished business" is often located in a physical rather than a verbal form. The child who cannot complete a gesture, who cannot finish an action, who leaves every activity half-done, is showing an interrupted gestalt, and the intervention is to help the child complete the action, physically, in the session.

**With adolescents**, the modification concerns the pace and the holding. The adolescent's contact with the field is generally good, and the challenge is the regulation of the affect that the contact brings. The developmental judgement is that the adolescent can tolerate a great deal of feeling provided the practitioner can stay present and provided the session has a structure that signals its boundaries.

**With young people in crisis**, the modification is substantial. The Gestalt work is contraindicated in acute crisis, and the practitioner must establish safety, predictability, and some control before any confrontation. Chapter 38 develops this.

## 6.3 The specific risk with young people

The specific risk of the confrontational techniques with young people is that a young person who has been criticised, shamed, or hit will experience the confrontation as a repetition of the familiar, and will respond with the compliant or defensive behaviour they have developed. The practitioner must therefore be attentive to the young person's developmental history of being confronted, and must calibrate the intervention accordingly. A young person who has been shamed will need to know the difference between a challenge and a humiliation before the challenge can be useful, and that knowledge comes from the relationship, not from the technique.

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# Further Reading for This Chapter

- Perls, F., Hefferline, R. C., & Paivio, A. (1991). *Gestalt therapy: Introduction and new developments*. Crown. — The primary theoretical text, and the source of the cycle of experience and the theory of the field.
- Perls, F. (1969). *Ego, hunger and aggression*. Real Books. — The short book on the developmental theory of the self, and the source of the "safely abandoned" concept.
- Ginger, S. (2004). *The gestalt approach in counselling and psychotherapy*. Sage. — The most useful contemporary clinical account.
- Clarkson, H. (1992). *The therapeutic relationship*. — A detailed treatment of the contact theory and the theory of the field.
- Levine, M., & Salter, J. (2016). *Principles and practice of gestalt therapy*. — A practice-oriented and well-illustrated account.
- Isbell, D., & enright, E. (2012). *The here and now of gestalt therapy*. — Short and clear; a good first read.


# 7. Modern Developments and Variants

## 7.1 The Chicago and Esalen schools

The version of Gestalt therapy that developed in the 1960s through the Esalen Institute, and the humanistic and encounter movements generally, differs from the New York school in three respects, and a student who reads only the popular literature will encounter the Esalen version and not the other.

**Contact becomes "authentic encounter."** The emphasis shifts from the structured analysis of the field to the quality of the encounter, with the "authentic" or "warm" dimension foregrounded and the confrontational techniques attenuated. The theory of contact is retained but the practice of confrontation is softened.

**The "safe place" is theorised.** The concept of the "safely abandoned" environment, in which the young person can risk the exposure of what is unfinished, is developed into an explicit precondition for the work. The theory of the field is retained.

**The emotional first is added.** The training methods of the Esalen school developed a capacity for working with affect directly, through a variety of experiential and body-oriented methods, that the New York school did not emphasise.

The two versions are not equally well supported by evidence, and the encounter literature of the 1970s produced a number of well-documented cases of harm to participants in intensive experiential programmes, several of them involving the inappropriate use of the confrontational techniques with vulnerable people. The student should be aware of this history, and should not treat an account of a training experience as a description of clinical practice.

## 7.2 The contemporary state

Gestalt therapy today is practised in a form that is generally more integrative than its history suggests, and a student in a contemporary clinic will find that most Gestalt-trained practitioners combine the contact theory and the here-and-now attention with techniques from other traditions. The theoretical core — the field, the contact, the unfinished gestalt, the awareness zones, the boundary — remains coherent and is used to

organise attention rather than to determine the method.

**Gestalt therapy with trauma.** The somatic and trauma-informed literatures have drawn heavily on Gestalt concepts, particularly the account of the boundary and the account of the interrupted cycle, and the "titration" idea — the principle that the practitioner's approach to the contact is titrated against the young person's capacity to stay in contact — is now a standard element of trauma-informed practice. Chapter 38 develops this.

**Gestalt in education and organisations.** The theory's account of the organism–environment field has been influential in educational and organisational theory, and the concept of contact in group process is widely used in group work with young people.

## 8. Evidence and Research Support

**The evidence position is limited, and the honest statement is short.**

**What exists.** A small number of controlled outcome studies of Gestalt therapy with adults, most of them from the 1980s and 1990s, reporting favourable outcomes in comparison to wait-list controls. A number of uncontrolled outcome studies and case series. A body of process research, largely from the 1970s and 1980s, on the here-and-now process, on confrontation, and on the therapeutic relationship in Gestalt therapy, conducted largely by the same research group and therefore not independent. And a substantial clinical literature, consisting chiefly of case material, that is theoretically rich and empirically unverified.

**What the systematic reviews conclude.** The reviews consistently rate the evidence for Gestalt therapy as limited, with small samples, heterogeneous outcome measures, high or unclear risk of bias, and considerable difficulty in obtaining adequate control conditions — a difficulty that is intrinsic to the method, since a Gestalt therapist's practice is a distinctive practitioner-in-the-field event that is difficult to reproduce or to control for.

**The comparison with other approaches.** There is no reliable evidence that Gestalt therapy is superior to other bona fide therapies for any condition, and the general equivalence finding of Chapter 6 applies. Where a specific claim is made — that the here-and-now confrontation is the active ingredient, that the empty chair is effective for specific purposes — the supporting evidence is a small number of studies with methodological limitations.

**The methodological problem, honestly stated.** The field's evidence problem is not principally a problem of technique. It is a problem of theory-testing: a theory that claims the practitioner is part of the field and that the present moment is the significant data is, in its own terms, extremely difficult to evaluate with the conventional research designs, because those designs are designed to isolate variables and to hold the practitioner constant, and the theory's central claim is that the practitioner cannot be held constant. A student who understands this will understand why the evidence is thin without concluding that the theory is empty.

**The relational findings.** The findings that support the theory are, as in most of the field, the relational ones. The consistency of the alliance–outcome relationship across therapies, and the finding that what matters is the young person's experience of being understood, are consistent with a theory whose central claim is about the quality of the encounter.

## 9. Strengths and Limitations of Gestalt Therapy

**Strengths.** It offers a clear and non-trivial account of how a person functions, in which symptoms are intelligible as attempted solutions and in which the person's resources are located in their own unfinished business rather than in an external supply of technique. Its emphasis on the body and on the present moment is well matched to developmental reality and to the somatic literature. Its account of the boundary and of differentiation maps precisely onto the developmental task of the second decade. It is unusually good at working with young people who have difficulty talking and who have difficulty with feeling, because it does not require either. And it provides the practitioner with a way of noticing what is happening in the room that other theories do not supply.

**Limitations.** The evidence is thin and the theoretical claims are difficult to test. The confrontational techniques are easy to misuse and have a documented history of misuse in training contexts. The theory's account of the person is developmental and individualistic in a way that is contestable, and it has been criticised for under-estimating the social and structural determinants of a young person's difficulty. And the theory's emphasis on the individual field can obscure the social field, in a way that the systemic and social justice approaches correctly identify.

## 10. Cultural and Ethical Considerations

**The cultural location of the field.** The Gestalt account is built on a specific set of assumptions that are worth naming: that the individual is the relevant field; that the boundary between self and environment is drawn by the individual; that a healthy development consists in the achievement of differentiation; and that the "peak experience" or the fully integrated gestalt is the goal. Each of these is contestable, and each is contestable in a way that has been tested by the cross-cultural literature. The achievement of differentiation as a developmental goal in particular presumes a family structure in which the young person is expected to become separate, and it does not travel well to contexts in which the family and the community are constitutively part of the person, in which the merging of self and environment is a form of belonging rather than a symptom.

**The ethical question of confrontation.** The technique of confrontation raises a serious ethical question with young people, and the question is not the practitioner's intention but the young person's experience of it. The ethical requirement is that the confrontation be delivered in a way the young person can distinguish from the confrontations they have been subjected to, which requires the relational conditions to be in place, and it is a requirement that the profession's codes do not spell out and that the theory does not address. A practitioner who confronts a young person who has never been treated with respect is not providing an experience; they are providing a repetition.

**The ethical question of the empty chair.** The empty chair technique, in which a young person is asked to address an absent person — who may be a parent who abused them — has obvious ethical dangers, and the danger is not mitigated by the theoretical account. The professional requirements are: the absent person must not be a current danger; the young person must have adequate support and adequate stabilisation; the practitioner must be competent in the technique and in the management of affect; and the young person must have consented to a procedure they understand, which is a real difficulty when the procedure is unfamiliar and evocative.

**The ethical value of the theory.** The ethical value of the approach should also be recorded. It is one of the few therapies in the field that takes seriously the young person's own body, their own voice, their own pace, and their own capacity for contact. It is also one of the few that does not require the young person to be

verbal, articulate, or introspective in order to benefit, which matters enormously for young people who are pre-verbal, neurodivergent, or in a state of arousal in which speech is unavailable.

## 11. Case Illustration: Samuel and the Chair

**The referral.** Samuel, eleven, is referred by a school because of "aggression and shutting down." The school reports that he is a bright child who does well academically, that over the last six months he has had three incidents in which he has shouted at staff and walked out, that in the incident three weeks ago he pushed a younger pupil and was excluded for two days, and that he has become withdrawn in class. His parents report that he "has always been a perfectionist and it is getting worse." There is a recent and unresolved parental separation, with the mother having moved out in June and the father sharing with his new partner.

**The first session.** Samuel sits with his arms folded and answers in monosyllables. The practitioner does not pursue the incidents. Instead she does the thing the theory prescribes: she attends to the here and now, and she says what she sees. "Your shoulders are up by your ears, Samuel. And you have moved the chair about two inches away from me since you sat down."

He looks up. He says: "Is that a thing you notice?" She says: "It is a thing I notice. And I noticed you check the door when you came in." He says: "I was checking if my dad was in the corridor." She says: "Was you expecting him?" He says: "No." A pause. "Yes."

**The developmental reading.** The practitioners' initial formulation is not a behavioural one. The theory's account of the contact interruptions suggests that Samuel is retroflexing: he is unable to direct an assertion at his father, and the energy has been turned back on himself and on the people in front of him — hence the shouting, the pushing, and the academic perfectionism, which is an activity in which the interrupted need for control is completed. The developmental reading adds that Samuel is eleven and in the middle of the separation, and that the separation is the interrupted gestalt: something that has not been completed, cannot be completed, and is being attempted at every point of contact.

**The empty chair.** The practitioner is cautious and follows a considered sequence. She establishes contact over three sessions before any technique, and in the third session she says: "There is a chair over there, and I would like you to pretend it is your dad. I want you to say the things you have not said to him."

Samuel does not want to. He says: "He'll just tell me to stop being dramatic." The practitioner does not argue. She says: "That may be true. What will you say to him anyway?" He says: "I'll probably cry." She says: "That would be all right."

He speaks for four minutes. The content is not what the practitioner expected and is much more important: it is not anger. It is a series of small practical questions — "Did I do the right thing on Tuesday?" "Is the house too small?" "Do you think she thinks I chose her?" — and the last of these is followed by a collapse into crying that lasts eleven minutes. The practitioner stays with it, does not interpret, and does not move towards reassurance.

**What the theory says about what happened.** The unfinished business was not the anger at the father. It was the question of loyalty: which parent the child belongs to, and whether the child is a traitor for continuing to live with the father. The child had been unable to ask the question directly, and the question had been driving the whole presentation. The task of the session was not to express the anger; it was to complete the contact that had been interrupted.

**The integration.** After the session Samuel is depleted and polite, and the practitioner says what she is going to do and why. "I am not going to do this every week, and I do not want you to do it again until you want to. We will talk instead." Over the following weeks they talk. The work with the parents — which the practitioner insists on, and which the Gestalt theory does not require — is the part that actually resolves the interrupted gestalt, because the question cannot be completed in the session; it can only be completed in the family.

**The critique of the case.** Two criticisms are worth making, and the practitioner makes both to herself. The first is that the empty chair was premature: Samuel had been in the service for three sessions, and the developmental literature suggests that a longer establishment of contact and a fuller assessment of the family situation should have preceded a technique of this intensity, particularly with an eleven-year-old in an unresolved separation. The second is that the technique was in some respects unnecessary, because the more direct route to the same material would have been for the practitioner to say, in the first session, "It sounds as though something is happening at home, and I'd like to hear about it" — which is an ordinary therapeutic intervention, and one that the here-and-now attention to the here and now had almost missed in favour of the more dramatic technical option.

**The teaching point.** The theory's value in this case was the formulation — the interrupted gestalt, the retroflexion, the question at the centre — and the value of the here-and-now attention was the observation of the two inches of chair and the glance at the door, which produced the material that the technically more sophisticated intervention would have been used to reach. And the practitioner who had stopped at the technique would have missed the parental work, which is where the case was actually resolved.

## 12. Key Terms

### Key Terms at a Glance

- **Gestalt** — The German term for a whole or configuration, borrowed to name the theory's central claim that the mind organises experience into wholes and that the tendency toward a completed and differentiated whole is the ground of growth.
- **Field** — The bounded region of activity in which the person and the environment are mutually determining; the unit of reality and of intervention in Gestalt therapy.
- **The organism–environment field** — The principle that the person and the environment are a single field of mutual determination, and that the practitioner's presence is constitutive of it.
- **Prägnanz** — The perceptual law stating that organisation tends toward the simplest, most stable, and most complete whole; the theoretical ground of the actualising tendency.
- **Figure and ground** — The organisational principle that a field is structured into a foreground element that is attended to and a background that recedes; the basis for the account of attention and of exclusion.
- **Here and now** — The total present situation of the therapeutic field, including the practitioner; the location of the most significant available data.
- **Contact** — The boundary event in which organism and environment act on one another, characterised by presentment, focusing, and full contact.
- **Contact interruption** — A disturbance of the contact process; includes premature closure, retroflexion, projection, confluence, and deflection.
- **Retroflection** — The turning back on oneself of a communication that should be directed outward; the most common contact interruption in clinical work with young people.
- **Confluence** — The dissolution of the boundary between self and environment in order to avoid the anxiety of the boundary; associated with a failure of differentiation.
- **Premature closure** — The completion of a gestalt before the contact is complete; a form of the "should" of the character structure.
- **Cycle of experience** — The sequence of approach, heightened awareness, contact, integration, and withdrawal; completed many times in ordinary life.
- **Unfinished business** — The persistence of an incomplete gestalt as a drive that organises the person's behaviour; the material of the therapeutic work.
- **Interrupted gestalt** — A gestalt whose completion has been prevented by fear, obstruction, or internal prohibition, and around which the person's behaviour is organised.
- **Three zones of awareness** — The zone of awareness, the zone of pre-awareness, and the zone of unaware material, used to locate where a person is attending.

**Key Terms at a Glance (continued)**

- **Excitation** — The deliberate slight disruption of the field that brings pre-aware material into focus.
- **Experiment** — The deliberate alteration of the field by the practitioner in order to create an unfamiliar experience and observe the young person's response.
- **Empty chair** — The technique in which a young person speaks to a chair representing an absent person, in order to complete an interrupted contact.
- **Hot seat** — The technique in which the young person and practitioner exchange positions, so that the young person experiences the interaction from the other side.
- **Gestalt prayer** — The personalisation exercise in which a statement of feeling is rephrased in the first person singular and addressed to the person to whom it belongs.
- **Dream work** — The technique in which a reported dream is worked through as a gestalt in the process of completion, element by element, in the first person.
- **Safely abandoned** — The state, in the field, in which the young person can risk exposure of what is unfinished; a precondition for the work in the Esalen formulation.
- **Titration** — The principle of matching the intensity of the practitioner's approach to the young person's capacity to remain in contact; widely used in trauma-informed practice.

## 13. Chapter Summary

Gestalt therapy descends from the perceptual psychology of the Berlin School, from the phenomenology of the Würzburg school, from Goldstein's neurological work, and from Perls' own wartime clinical experience with acutely disturbed soldiers. Its core propositions are that perception is organised into wholes, that a person is an organism–environment field in which the practitioner is a constitutive part, and that there is a tendency toward a coherent, completed, and differentiated whole.

The clinical consequences are direct. A symptom is an uncompleted gestalt, an attempt at a solution rather than a failure. The most significant data is the present moment. Contact with the environment is the ground of growth, and its interruptions — premature closure, retroflexion, projection, confluence, and deflection — are the forms a young person's difficulty takes in the field. The account of the boundary supplies a developmental language for the adolescent's task of differentiation.

The technique repertoire — the empty chair, the hot seat, the gestalt prayer, the dream work, and the confrontation of the here and now — is derived from the theory, and each carries specific indications and cautions, particularly with young people who have been shamed, who are in crisis, or who are at risk of flooding.

The developmental fit with young people is good: the emphasis on the body, the attention to the present, and the account of differentiation map onto the second decade's central tasks. The evidence is limited, and the limitation is partly intrinsic, because a theory that claims the practitioner is part of the field is difficult to test with designs that hold the practitioner constant. The case of Samuel illustrates the theory's power — the formulation of the interrupted gestalt and the retroflexion, and the here-and-now observation of the two inches of chair — and also its risks, since the empty chair was arguably premature and the work with the

parents, which the theory does not require, was where the case was actually resolved.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. State the three propositions of Gestalt theory that come from the Berlin School, and give the clinical translation of each.
2. Explain the account of the field, and state three clinical consequences that follow from it.
3. Describe the five forms of contact interruption, and give an adolescent example of each.
4. Explain the cycle of experience and the concept of the interrupted gestalt, and state the three consequences of an interrupted gestalt.
5. What does the theory mean by "here and now," and why is the popular version of the phrase misleading?
6. Compare the Gestalt account of symptom formation with the behavioural account, and state the practical difference in the work.
7. State the indications and the cautions for the empty chair technique, and explain why the cautions are more substantial with young people.
8. Why is the Gestalt theory particularly difficult to evaluate with conventional research designs?
9. In the Samuel case, what did the two inches of chair tell the practitioner that the referral did not, and what would have been missed by going straight to the empty chair?
10. What is the risk of the confrontation technique with a young person who has been shamed, and what does the theory not address about it?

## 15. Practice Activities

**Practice and Application**

### Activity 12.1 — The Here-and-Now Observation (90 minutes, triads)

One student plays a fourteen-year-old who is being "worked on" by a well-meaning adult in an office; one plays the practitioner; one observes. The practitioner's only task is to notice and to state what they observe in the field — the body, the distance, the speed of the speech, the glances, the silences — without interpretation and without advice. The observer logs each observation and the young person's response. The exercise is a great deal harder than it sounds and it develops the most transferable skill in the theory.

### Activity 12.2 — Contact Interruption Mapping (75 minutes, individual)

Take a young person you know well and write a paragraph for each of the five contact interruptions, in the form "when X happens, he/she tends to..." — for example, "when she is angry with her mother, she apologises to her instead, and then withdraws from everyone for the rest of the evening, and then apologises to them as well." The exercise makes the interruptions concrete, and the retroflection pattern in particular is often recognisable in a way the student does not expect.

### Activity 12.3 — The Technique Triage (90 minutes, small groups)

Each group takes one of the four principal techniques and works out: the precise formulation of the case in which it is the first choice; the formulation in which it is contraindicated; the developmental modification for a seven-year-old, a twelve-year-old, and a seventeen-year-old; and the practical preparation required before using it with an adolescent. The contraindication column is the most important part of the exercise and the one students are most likely to fill in incorrectly.

## 16. Annotated Reading Pathways

**For the theory.** Read Perls, Hefferline, and Paivio (1991) for the field theory and the cycle of experience, and Perls (1969) for the developmental theory of the self and the concept of the safely abandoned place. The latter is short and considerably more readable than the former.

**For the clinical application.** Read Ginger (2004) for the contemporary clinical account, and read the contact theory chapters in Clarkson (1992) for the detail. A student who intends to use any of the techniques should read them in one of these two books before using them with a person.

**For the developmental application.** Read the somatic and trauma-informed literature that has absorbed Gestalt concepts, particularly the material on titration and on the window of tolerance, and see how the theory's boundary concepts have been operationalised for work with young people.

**For the history and the cautions.** Read the critical literature on the encounter movement and on the misuse of experiential techniques with vulnerable people. The history is a real part of the theory's inheritance, and a practitioner who knows it will be more careful with the confrontation techniques than one who does not.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Gestalt Therapy: Awareness, Contact, and the Here and Now** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is perls, Foulkes, and the experimental phenomenology of the counseling session. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is

helping rather than merely continuing. The organising focus for this part of the course is awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The mechanism to keep in view is how safety, attention, expression, and relational responsiveness allow experience to become available for reflection and choice. The characteristic professional risk is pressuring disclosure, treating emotional intensity as progress, or neglecting risk, family context, and practical action. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in individual sessions where trust, voice, and self-understanding are central, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Kofi is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Kofi's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Kofi explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The first intervention is deliberately modest. Kofi chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Kofi has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Kofi can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Gestalt Therapy: Awareness, Contact, and the Here and Now

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Gestalt Therapy: Awareness, Contact, and the Here and Now** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Gestalt Therapy: Awareness, Contact, and the Here and Now** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

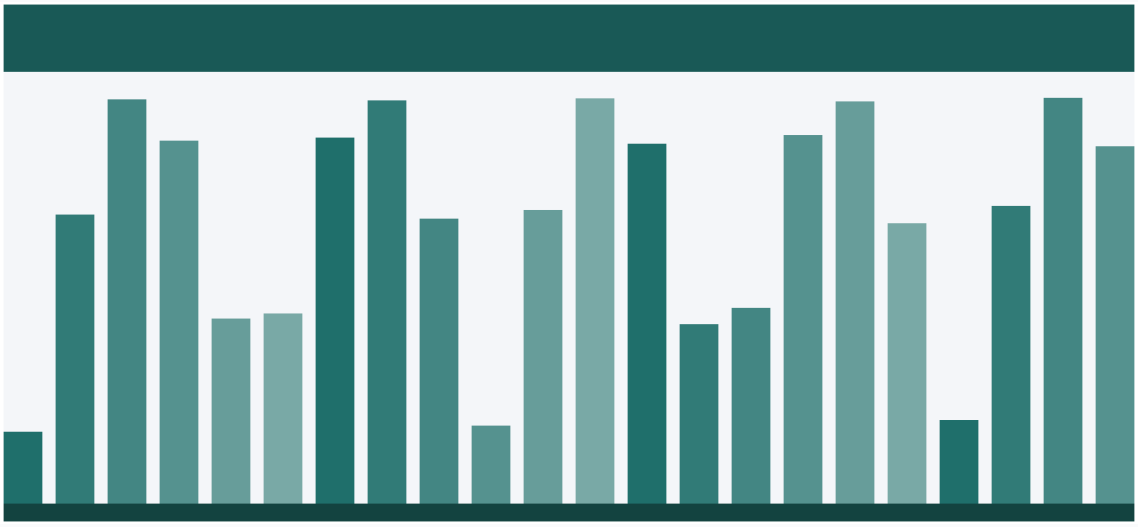
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Gestalt Therapy: Awareness, Contact, and the Here and Now**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Gestalt Therapy: Awareness, Contact, and the Here and Now to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 13

# Focusing-Oriented Therapy and the Felt Sense

Gendlin's experiential method and the therapeutic use of bodily felt meaning.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe Eugene Gendlin's intellectual development and explain the events that led him to develop the theory of the felt sense.
- Explain what the felt sense is, how it differs from emotion and from bodily sensation, and why Gendlin claimed it is the foundation of psychological change.
- State the six principles of focusing and give a clinical example of each.
- Describe the five-step focusing procedure and the role of the practitioner in each step.
- Explain what the theory has to say about bodily symptoms and their relation to psychological difficulty.
- Apply focusing with children and adolescents, and specify the developmental modifications required.
- Describe the modern developments, including low-resourced focusing and the integration of focusing with other modalities.
- Evaluate the evidence for focusing and for the felt sense construct.

## Chapter Overview

Eugene Gendlin's discovery was made in the 1940s, in the course of a very unusual set of circumstances, and the story of how it was made is itself instructive. Gendlin was a member of a research team at the University of Chicago studying what happened to patients in psychotherapy, and he noticed something that the rest of the team did not: that a small number of patients reported a particular kind of experience, describable as a wordless, bodily, whole, and yet indefinable sense of their own situation, and that these patients improved, and that the improvement correlated with their capacity to access and to stay with that experience.

The finding was so odd that it was dismissed by the research team. Gendlin spent the next two decades establishing what he had found, and the result was a theory of experiencing that is one of the most important and least known contributions to the field. The theory holds that the felt sense — the bodily, pre-verbal, whole, and undefinable experience of the current moment — is not a symptom, or a by-product, or a doorway to the "real" feelings, but is itself the location in which psychological change occurs. A person who can access and stay with their felt sense has access to the resources for their own change, and a person who cannot is, in a sense, cut off from their own living.

The theory is a natural companion to the somatic and trauma-informed work of Part VI, and it is also a distinctly humane technology: it is teachable in a few sessions, it works with people who cannot articulate, and it respects the young person's own experience in a way that no technique requiring verbal insight can.

This chapter presents the theory in its intellectual context, describes the felt sense and the experiential process, sets out the principles and the procedure, and addresses the developmental and ethical questions.

## 1. Historical and Intellectual Context

### 1.1 The research project

Gendlin trained as a philosopher at the University of Chicago, where he was a member of the philosophy faculty, and he was a colleague of Carl Rogers, whose client-centred work was then in its productive middle period. In 1946 Gendlin was recruited into a research project funded by the Rockefeller Foundation, which was studying the process of psychotherapy at the University of Chicago's Counseling Center. The project was examining, among other things, what made some clients improve and others not, and it was a period in which the research team was deeply invested in the client-centred account of the relationship.

Gendlin noticed that a particular group of clients, a small minority, reported a distinctive experience. They described something they could not put into words, which was located in the body, which was a whole rather than a part, and which was somehow the experience of their own situation rather than of any particular content. These clients were the ones who improved. The finding did not fit the project's framework, and the research team's published conclusions did not include it. Gendlin was, by his own account, told that he had found nothing.

### 1.2 The next two decades

Gendlin's subsequent work was an extended effort to establish the phenomenon empirically and to develop a theory from it, and it is one of the more striking examples of a researcher refusing to abandon a finding that his professional community did not accept. Three strands developed.

**The phenomenological enquiry.** Gendlin and colleagues used phenomenological methods — detailed description of the experience of the moment — to characterise the felt sense and to describe its structure, which he characterised as a triple structure: a bodily location, a sense of a whole, and a sense of a direction or vector of movement. The third element is the one that is most often missed and it is the most useful clinically: the felt sense has a *movement*, an orientation, a sense of where the experience is going, and a practitioner who can help a young person track that movement has found the therapeutic process.

**The experiments.** Gendlin's "focusing experiments" used a single-subject design in which participants were given a specific bodily task — typically a set of sentence stems referring to body sensations, such as "the part of me that..." — and were asked to find the bodily location and the quality of their experience. Participants who located the bodily sense of their situation in the specific way Gendlin described showed measurable changes in self-reported emotional state and in their sense of the situation, and the changes exceeded those produced by other conditions, including conditions in which participants were asked to think about their feelings in a more conventional manner.

**The development of the method.** Focusing was developed as a client-led practice in which the practitioner does not guide the process but creates the conditions in which it can occur.

### 1.3 The tradition

Gendlin's position in the field is unusual: he was a philosopher by training, a researcher by temperament, and a clinician by practice, and he is respected in the humanistic tradition, in the somatic field, in the contemplative traditions, and in the phenomenological philosophy. The theory's specific contribution has been greater than its diffusion, and a student who knows it will find that it supplies a missing element in several of the other theories in this book.

## 2. The Theory of Experiencing

## 2.1 The felt sense

The **felt sense** is the living, bodily, whole, and largely undefinable experience of one's current situation. Gendlin's own description is characteristically precise: it is a sense of the whole of one's situation as it is being lived, which is located in the body, which is felt more than thought, which is vague and difficult to put into words, and which is nevertheless distinguishable from any particular emotion, sensation, or thought.

Four of these features require emphasis, because each distinguishes the felt sense from something it is often confused with.

**It is a sense of the whole.** The felt sense is not an emotion, and it is not a localised sensation. A young person who says "I feel anxious" is reporting an emotion, which has a name and a phenomenology that the emotion recognition literature has studied. The felt sense is what it is like to be in that state, in a way that is more general than the emotion and that includes an implicit sense of the situation and of what is going on. It is the felt "being-in-it".

**It is bodily.** The felt sense is located in the body, and it is experienced as a physical sense of the situation, often in the centre of the body, though not always. Gendlin's experimental method exploited this, and it is the empirical anchor of the theory.

**It is pre-verbal.** The felt sense is typically not in words, and the attempt to put it into words usually results in a change of experience, in which the living quality is replaced by a static concept. This is the practical reason why the focusing method requires that the young person not talk while they are in the felt sense.

**It is undefinable but not vague.** The felt sense is precisely felt; it has a quality that the young person can distinguish from other qualities. It is not analysable, but it is not nebulous either, and a practitioner who is learning the method will find that they can be told when a young person is in the felt sense and when they have moved into intellectualisation.

## 2.2 The claim

Gendlin's claim is the theory's central and most radical proposition, and it should be stated in its strongest form:

**Psychological change happens in the felt sense, and the felt sense is the locus of the person's own resources for change.**

The implication is that a person does not need to be given anything, and does not need to be interpreted, and does not need to acquire a new technique. A person who can access their felt sense can find their own answers in it, and this is why the practitioner in focusing does not direct the process. Gendlin described the practitioner as providing a "special kind of listening" — an attention to the whole of the client's experience, including the bodily and the wordless, and a stance of "being with" rather than of interpreting.

The claim is also the reason the theory is difficult to place in the field's conventional taxonomy. It is a humanistic theory in its respect for the young person's autonomy, a somatic theory in its respect for the body, a phenomenological theory in its epistemology, and a process theory in its account of what therapy is.

## 2.3 Implications

Several implications follow, and they are clinically substantial.

**Symptoms and the felt sense.** A physical symptom, in this account, is the felt sense of a situation that has not been resolved, localised and held in the body. The theory therefore treats somatic complaints as meaningful rather than as either medical or imagined, and it does so in a way that is compatible with the contemporary somatic literature without dismissing the need for medical assessment.

**The primacy of experience over explanation.** A young person who understands what is happening to them may nevertheless be unable to change it; and a young person who does not understand may be able to change. The theory privileges the second case, and its claim is that the felt sense is where change is possible.

**The therapist's stance.** The therapist's role is to provide a specific quality of attention, not to interpret, advise, or direct. This places focusing in a lineage with the person-centred and the phenomenological traditions.

**Bodily attention as the route to emotional processing.** A young person who cannot talk about feelings can be helped to find the bodily sense of the situation, and the emotional work becomes possible from there. This is the theory's most important practical contribution to work with young people.

# 3. The Experiential Process

## 3.1 The structure of the process

Gendlin's account of the experiential process, developed with the help of the philosopher Eugene Boss and others, is the theoretical core of the theory, and it is a model of a mechanism.

The process has three components.

**A felt sense of the whole.** The person's experience of their situation as a whole, in the body, undefinable in words.

**The sense of a shift.** The experience that something is different, that something has changed, that the situation is not as it was. The shift is felt as a movement, a directional change, a sense of lifting or opening or settling, and it is the process element of the experience.

**A change in the sense of the situation.** The result: the person experiences the same situation differently, because the way in which they hold it has changed. Gendlin described the result as a "different sense of the situation" rather than as a new thought or a new feeling.

## 3.2 The process and the self

The theoretical claim connecting the three is that the "self" is not a thing but a process, and that the felt sense is the medium in which the process occurs. Gendlin's account of the self is a process model, and it converges with the person-centred formulation of the person as a process and with the contemporary "person as process" position of Schwartz and colleagues discussed in Chapter 10.

## 3.3 The relationship to other theories

The connections are instructive. The theory's account of the felt sense as the locus of change corresponds to the somatic tradition's account of the body as the site of the affective and the trauma-relevant. Its account of the practitioner's stance as a particular quality of listening corresponds to the person-centred account of empathic understanding. Its account of the self as a process corresponds to the contemporary person-as-process formulations. And its account of the "edge" — the boundary between the felt sense and

the conceptual — corresponds closely to the concept of the window of tolerance that is central to the somatic and trauma-informed work of Part VI.

## 4. The Six Principles of Focusing

Gendlin's six principles are the practical core of the method, and a student should be able to state each and give an example of its application with a young person.

**First principle: the felt sense is distinct from the content of the problem.** The young person's difficulty is not the same thing as the felt sense of the difficulty. A young person who is frightened of the school corridor may have a felt sense that is not about the corridor at all, but about something in the corridor that resonates with a long-standing sense of being exposed and judged. The practitioner does not work with the content; the practitioner helps the young person to find the felt sense, and the content changes from the changed sense.

**Second principle: a bodily location.** The felt sense is located in the body, and finding the location is the first practical task. A young person who cannot find a felt sense in a specific bodily place is usually trying to think the feeling rather than find it, and the practitioner's task is to redirect the attention to the body and to the particular.

**Third principle: the felt sense is wordless.** The young person does not need to describe it in order to have it, and describing it in words tends to move the experience out of the felt sense and into conceptualisation. The practitioner must therefore tolerate the young person's silence during a focusing session, and must be able to tell the difference between a productive silence and a withdrawal.

**Fourth principle: the shift is the process.** The work is to find and to stay with the shift — the sense of something changing — rather than to analyse the situation. This is the most counterintuitive and the most important of the principles, because the young person's natural tendency in the session is to analyse, to explain, and to solve, and the whole of the practitioner skill lies in holding the attention on the felt quality.

**Fifth principle: there is a bodily felt sense of a process, and a direction.** The felt sense has a direction, and a practitioner who helps the young person track the direction of the movement is doing the most therapeutic thing available. Gendlin's formulation of the "felt sense of a shift" includes the possibility of the direction being blocked, and the work then is to find what is blocking it, which is often a felt sense of a different kind.

**Sixth principle: there is a difference between the felt sense and the conceptual.** The felt sense is pre-verbal and the conceptual is verbal, and the practitioner must be able to help the young person move back from the conceptual to the felt. The "edge" is the term for the boundary between them, and the "inner edge" is the felt sense itself.

## 5. The Focusing Procedure

### 5.1 The five steps

The standard focusing procedure, which a student should be able to describe, has five steps, and they may be practised in a single session or across several.

**Step 1: Set the frame.** The practitioner establishes the conditions: a quiet setting, a comfortable position, and a statement of what will happen — that the practitioner will listen and will not give advice, and that the young person will be doing the work. The framing is a precondition, because the theory's claim is that the young person has the resources and that the practitioner's task is to make them available.

**Step 2: Find a bodily location of the problem.** The young person is invited to locate the difficulty in the body, using a set of sentence stems if helpful: "The part of me that..." "The place in my chest where..." "The tight feeling in my..." This step is frequently the easiest and frequently produces the first significant shift, because the young person discovers that the difficulty has a location and a size.

**Step 3: Find the felt sense.** The young person is invited to find the quality of the experience, distinct from the emotion and from the thought. The classic instruction is: "Instead of telling me how it is, could you be aware of how it feels?" The practitioner's task is to listen with a particular quality of attention — attending to the whole, including the wordless, and listening for the shift when it occurs.

**Step 4: Stay with the felt sense.** The young person stays with the felt sense, and the practitioner stays with the young person, attending to the direction of the movement and to any sense of blockage. Gendlin described this as a state of "patient waiting" in which the practitioner holds a quality of attention that is warm, wide, and free of interpretation.

**Step 5: Invite the shift.** When the shift occurs, the practitioner may say: "Something is different, isn't it?" or "There's a change there." The acknowledgement of the shift is itself part of the mechanism, because it brings the change into the verbal awareness of the young person and consolidates it.

### 5.2 The practitioner's role

The practitioner's role in a focusing session is distinctive and is often the hardest part of the method for a trainee. The practitioner listens, attends to the whole of the experience, listens for the wordless, tracks the direction of the felt sense, waits patiently, and does not interpret. The temptation to interpret — to say what the felt sense means, to make it meaningful, to connect it to something in the young person's history — is strong, and it is the principal way the method is spoiled.

Gendlin's description of the practitioner's listening as a "special kind" of listening is worth unpacking: the attention is not on the content of what is said but on the quality of the experience as it is lived, and it includes an attention to the body and to the shifts.

### 5.3 Variants

Several variants of the procedure exist, and a student should know them. **Intensive focusing** is a form in which a practitioner guides the process more actively, and it is used with people who have difficulty finding the felt sense on their own. **Low-resourced focusing** is a form in which the process is conducted in a single session and with minimal framing, and it is the most practical version for use in a school or community service with a limited number of sessions. **Focusing with a specific problem** in which the felt sense is located in relation to a particular concern. And **focusing as a practice rather than a technique**, in which the young person is taught the method for their own use, which is the most durable form and the one most consistent with the theory.

## 6. Focusing with Children and Adolescents

## 6.1 The developmental fit

The developmental fit of focusing with young people is very good, and it is good for reasons that are specific rather than general.

**It does not require verbal sophistication.** The method works with a young person who cannot articulate, and the age-related limits on abstract verbal processing described in Chapter 9 are not obstacles to it. A seven-year-old can locate "the tight bit" in their chest, and the process will work.

**It respects the body's primacy in emotional processing.** The somatic and developmental literatures describe the body as the primary site of the young person's emotional life, and the contemporary consensus in developmental neuroscience is that the body's representations of emotion are available in awareness before the verbal ones are. The focusing method is, in effect, a technique for working with the earliest and most accessible level of emotional representation.

**It respects the young person's autonomy.** A young person who is invited to find their own bodily sense and to follow it is being given an unusual degree of control in a setting in which they usually have very little, and the developmental literature suggests that the experience of control is itself therapeutic.

**It is compatible with play.** Focusing has been adapted into play-based work with young children, and the "felt sense" can be located through sensory and imaginal play rather than through attention to the body, which is developmentally appropriate for children whose bodies are not yet a reliable source of self-attention.

## 6.2 The developmental modifications

**With younger children (approximately 4–8)**, the method is conducted through play, drawing, movement, and imaginal work, and the "felt sense" is located as a place, a colour, a texture, a size, or a character. The words "felt sense" are not used. The shifts are described in movement and image rather than in sensation.

**With pre-adolescents (approximately 8–12)**, the method is conducted in a more recognisable form, with body scanning and with the sentence stems, and the practitioner can introduce the vocabulary of the felt sense, which many children find liberating because it names a private experience that they had not known was shareable.

**With adolescents**, the method is at its most effective, and there is a specific clinical use that should be named: **the method of the felt sense is one of the fastest available routes to a young person's own resources in the presence of an emotional activation that has made verbal processing impossible.** A young person in acute distress who cannot speak can be helped to find the bodily sense and to follow it, and the shift that follows is frequently the first moment of relief in the session.

## 6.3 The specific cautions

**A young person in acute crisis** may be unable to stay with a felt sense, and the attempt may increase arousal. The developmental and clinical judgement is that the felt sense is not accessible to a young person who is flooded, and that stabilisation work must come first.

**A young person with a history of dissociation** may find the attention to the body's interior extremely difficult or frightening, and the somatic literature is clear that this requires careful preparation and pacing. The instruction "go inside your body" is a serious one in a young person who has learned to leave it.

**A young person who has been told that their body is wrong**, for example in the context of a disability, chronic illness, trauma-related body symptoms, or a body that has been commented on, may have a complicated relationship with bodily attention, and the practitioner must proceed with great care.

**A young person in a cultural context in which bodily attention is not the ordinary mode of processing feeling** may find the method strange or inappropriate. Chapter 48 addresses this.

# 7. Modern Developments

## 7.1 Low-resourced focusing

The most practically significant development has been **low-resourced focusing**, developed by Gendlin and colleagues in the 1990s in order to make focusing practicable in settings with limited time and limited resources. It consists of a single session in which the practitioner guides the process directly, using a set of structured instructions, and it has been used in a variety of settings including schools, primary care, and brief therapy services. The research literature on it is small but growing, and it is the variant most likely to be usable by a student in a placement.

## 7.2 Focusing in the somatic and trauma fields

The theory has been considerably more influential outside the person-centred tradition than within it, and its principal influence has been on the somatic and trauma-informed fields, where the concepts of the felt sense, the inner edge, and the direction of the felt shift have been absorbed into the practical vocabulary of stabilisation and resourcing work. Chapter 40 develops this application in detail, and the student who knows the focusing theory will find the stabilisation protocols considerably easier to understand.

## 7.3 The contemplative connection

The theory's structure — the attention to the whole, the wordless, and the direction of the movement — is closely parallel to the account of attention in several contemplative traditions, and the contemporary literature on mindfulness has drawn the parallel explicitly. The parallel is instructive for two reasons. It shows that the phenomenon Gendlin identified in a psychotherapy setting in 1946 has been independently described in a great many traditions, which is a substantial piece of evidence for its reality. And it shows that the differences are as instructive as the similarities: the contemplative traditions are systematic about the discipline of attention over years, and they generally aim at a change in the person rather than at the resolution of a particular situation.

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# Further Reading for This Chapter


- Gendlin, E. T. (1996). *Focusing-oriented psychotherapy: The method and its clinical application*. Guilford. — The primary clinical text, and the fullest statement of the theory.
- Gendlin, E. T. (1978). *Focusing*. Everest House. — The original practical manual; short, clear, and still the best introduction.
- Gendlin, E. T. (1990). *Experiencing: The child's discovery of self*. — The developmental application, written for parents and teachers, and full of practical material.

- Madigan, S., & Brazier, D. (2012). *The body in focus: Developments in focusing and beyond*. — The contemporary development, and the best source on low-resourced focusing.
- Paola, E., & Rosso, S. (2013). *Exploring the felt sense*. — A research review.
- Rice, L. N. (2015). *Feeling in the body: Exploring the felt sense*. — An accessible introduction.

# 8. Evidence and Research Support

## 8.1 The experimental foundation

Gendlin's original experiments, and the subsequent replication work, constitute a real body of evidence, and a student should be able to state it accurately.

**The design.** In the standard experimental procedure, participants are briefly instructed in a specific bodily task and then asked to find a bodily location and a quality of their current experience, in the manner described in the principles. Control conditions include instructions to think about the problem in a more conventional manner, to analyse the emotions involved, or to do nothing. Outcomes are measured before and after by self-report, and in some studies by physiological measures.

**The findings.** Participants who located the felt sense in the manner Gendlin described showed greater improvement in their reported emotional state and in their sense of the situation than participants in the control conditions, and the effect was present in the single-session design. The effect size in the original and replicated studies is moderate, which is a substantial effect for a single-session intervention with no practitioner involvement, and it is one of the more robust single-session effects in the psychotherapy literature.

**The limits.** The studies were conducted with university students, in the 1970s and 1980s, with self-report outcome measures, and the research group that produced them is small. The effect has been difficult to replicate in clinical populations, and the standard criticism — that the effect may be a expectancy effect, since the procedure is unusual and the participants are being given something they have not had before — has not been fully answered, although the presence of the comparison conditions is designed to control for it. The studies are nonetheless among the more interesting in the field, and the finding that a bodily attention task produces a measurable change in emotional state is a real and useful thing to know.

## 8.2 The clinical outcome evidence

The clinical evidence for focusing as a therapy is considerably thinner than the experimental evidence for the felt sense task, and the two should not be conflated.

A small number of controlled outcome studies of focusing therapy, mostly from the 1980s, reported favourable outcomes, particularly with the presenting problems of trauma, anxiety, and depression, and a small number of studies have compared focusing with client-centred therapy with equivalent outcomes. A substantial clinical literature exists in the form of case material, and the theory's own tradition is heavily case-based. The reviews consistently rate the clinical evidence as limited.

## 8.3 The somatic and trauma literature

The most substantial evidence for the theory in practice comes from the somatic and trauma-informed fields, in which focusing-derived concepts have been widely adopted, and in which the associated

techniques — grounding, resourcing, and the "window of tolerance" work — have a considerably stronger evidence base than focusing itself. The student should be careful to note the direction of the borrowing: the somatic field has adopted focusing's concepts and produced evidence for a broader set of practices, and the evidence for the specific focusing method is weaker than the evidence for the stabilisation work in which its concepts are embedded.

### 8.4 What is weak or absent

**The felt sense as a measurable construct.** The construct has resisted reliable operationalisation, and the measures of experiential processing used in the research are of variable quality. The theory's central claim — that the felt sense is the locus of change — is difficult to test, for the same reason that the Gestalt theory is difficult to test: a claim about a first-person process resists the conventional designs.

**The developmental evidence.** The evidence for focusing with children and adolescents specifically is very thin, notwithstanding the theoretical plausibility of the developmental applications and the long history of the practice with young people. The applications to younger children are widely described and poorly studied.

**The comparative evidence.** There is no reliable evidence that focusing is superior to other bona fide therapies for any condition.

## 9. Strengths and Limitations of Focusing-Oriented Therapy

**Strengths.** It identifies a phenomenon that is real, that is common, that is accessible without training, and that most therapies neglect. It provides a technique that works with young people who cannot talk, and the technique is unusually well suited to the somatic presentations that dominate contemporary practice. It is teachable: a practitioner can learn the method in a small number of sessions, and the low-resourced variant is usable in a school or community service with a single session. It respects the young person's autonomy more fully than almost any other method in the field, since the young person is doing the work and the practitioner is providing a quality of attention. It has a real experimental foundation, which is unusual for a humanistic technique. And it converges with the somatic, trauma-informed, and contemplative literatures, which makes it an unusually well-supported position in a field that is currently converging on somatic and body-based practice.

**Limitations.** The clinical outcome evidence is thin, and the theory's central claim is difficult to test. The method requires a practitioner who can tolerate silence and wordlessness, which is a demanding skill and is not developed in most training programmes. It offers little in the way of structure, and a young person who requires structure — which is many young people, and most young people in crisis — will not find it sufficient. The method's assumption that the young person has access to a felt sense is not always true, and the practitioner needs a way of working with a young person for whom it is not accessible. And the theory's account of the person, though empirically anchored, remains a first-person account, and it says relatively little about the social and structural determinants of a young person's difficulty.

## 10. Cultural and Ethical Considerations

**The body and cultural difference.** The theory's foundational claim is that the felt sense is accessible to the person, and this is not culturally universal. In cultural contexts in which emotion is located in the head or in the heart in a distinctly verbal way, in which the body is not treated as a source of knowledge about the self, in which physical expression is heavily regulated, and in which the boundary between self and environment is understood differently, the instruction to attend to a bodily felt sense may be experienced as strange, or as an invitation to a form of self-exposure that is not culturally sanctioned. The culturally responsive practice is to ask the young person how they know what is going on with them, and to follow their answer.

**The risk of the body's authority.** The contemporary somatic literature has been critical of the assumption that the body is an unambiguous source of truth about the person, and the focusing theory's claim that the felt sense is the location of the change carries this assumption. A young person who has been told for years that their body is unreliable, that their symptoms are "all in their head," or that their trauma responses are exaggerated, will have learned to distrust their own bodily signals, and the method's first task may be the restoration of that trust rather than the discovery of a felt sense.

**The ethical value of the method.** The ethical value of the method should also be stated, because it is considerable. It is a method that gives a young person access to their own resources without requiring them to disclose a narrative, to accept a formulation, to comply with a programme, or to become verbally articulate. It is a method in which the young person is the authority and the practitioner is the facilitator, which matters enormously with young people who have been told all their lives what they are doing wrong. And it is a method that can be used with a young person who has not yet trusted an adult enough to talk, which is frequently where the work needs to begin.

**The ethical requirement of the practitioner as a container.** The method places a substantial demand on the practitioner, and the demand is partly emotional. A practitioner working with a young person's wordless experience is exposed to a good deal of affect, and the practitioner must be able to sustain their own congruence while remaining available. The ethical requirement is that the practitioner attend to their own state, seek supervision, and not undertake this work with a young person if their own resources for it are depleted. This is a requirement that the professional codes express only obliquely, and that the theory does not express at all.

## 11. Case Illustration: A Boy and the Weather

**The referral.** Arjun, twelve, is referred by his school for "anger and withdrawal." He has had three suspensions in a term, is failing two subjects, and his form tutor reports that he "shouts" and then "goes completely silent" and "will not look at anyone." The mother reports that he has not spoken to his father in three weeks following an argument. There is no history of mental health treatment.

**The first session.** The practitioner — who has been trained in focusing and works in a school-based service with a low-resourced format — begins with a short piece of grounding, and then asks Arjun where in his body he notices the difficulty. He does not answer for a while and then says, "My chest." She asks what it is like. He says, "Like a storm." She asks him to stay with it for a moment and to notice whether it is changing. He says: "It doesn't stop."

**The low-resourced protocol.** The practitioner uses the structured low-resourced instructions. She asks him to find the place exactly, and he points to the middle of his chest. She asks him to describe the quality, and he says it is "like the weather before rain" and "it goes black." She asks him to stay with it. There is a long silence of perhaps forty seconds, and the practitioner says nothing. Then he says: "It's lighter." She says:

"Tell me more about that." He says: "It's like the sky after a storm. Still wet but you can see."

**The shift.** The practitioner does not say "what happened?" and does not ask about the argument with his father. She stays with the felt sense, and she asks him to stay with the lightness. He does, for several minutes, and during that time she observes that his posture changes, that his breathing changes, and that his face changes. When the session ends she asks him to notice what has changed, and he says: "I don't know. But it's different."

**The second session.** Arjun comes back and, unprompted, says: "I've been thinking about the storm thing." The practitioner asks him what he has been thinking. He says: "I think it's about waiting. At home it's always waiting. Waiting for my dad to come home, waiting for my mum to come back from work, waiting for him to stop being angry. And I'm waiting too."

**The formulation.** The developmental formulation is now available. Arjun's "storms" are the bodily representation of a life organised around a state of suspended waiting, and the waiting is the interrupted gestalt, or, in the language of this chapter, the felt sense of a situation that has not changed and cannot be changed by him. The suspensions at school are the behavioural form of the same state. The anger and the silence are the two poles of his management of an intolerable and unchangeable situation.

**The intervention.** The intervention is developmental and practical, and it follows from the formulation rather than from the technique. Arjun's life contains one adult — his mother — who works nights and one — his father — who is often angry. The practitioner arranges for a conversation with the mother and, with Arjun's agreement, with the father, and the conversation is about the household's schedule, the waiting, and the specific question of what happens at home in the evenings.

The focusing work itself is continued, and the specific use of it is that it gives Arjun a way of experiencing the difference between the storm and the sky after the storm. The practitioner teaches him a brief version of the method — which the theory predicts will be the most durable form of the intervention — and he uses it at home. He tells his practitioner, in the sixth session, that he used it in the car on the way to school, and that his dad had been shouting about something else and "I did the weather thing in my head and it went light and I didn't have to shout back."

**The developmental qualification.** The practitioner is careful about the sequence. She does not attempt the parental work until the relationship can carry it, and the parental work is not a confrontation of the father but a conversation about the timetable. The developmental literature is clear that the adolescent's renegotiation of the attachment relationship requires that the adult be available and reliable, and that the confrontation of a parent's behaviour before that has been established is likely to be experienced as a repetition.

**The ethical dimension.** Two ethical requirements were engaged. The first is the duty of care in relation to a family system in which a twelve-year-old is waiting for a parent to stop being angry: the practitioner has a duty to consider whether the household conditions constitute a safeguarding concern, and to raise it if the assessment indicates it. She assesses and concludes that they do not, but she records the assessment and the reasoning. The second is the consent question: the parental work requires Arjun's agreement to its shape, and his agreement was sought explicitly, and the practitioner was prepared for it to be refused, in which case she would have continued with the individual work and the school work.

**The teaching point.** The technique produced the entry into the material, and the theory produced the formulation, and the developmental frame produced the intervention. None of the three was sufficient alone. A practitioner who used the focusing technique without the developmental frame would have had a boy who could access a felt sense and nothing to change in his life. A practitioner who had the developmental frame

without the focusing technique would have had a family meeting with a boy who could not say what was happening to him.

## 12. Key Terms

**Key Terms at a Glance**

- **Felt sense** — The living, bodily, whole, and largely undefinable experience of one's current situation; the location in which Gendlin claimed psychological change occurs.
- **Experiencing** — Gendlin's term for the whole, live process of a person's moment-to-moment experience, as distinct from the conceptual or verbal level.
- **Experiential focusing** — The process of attending to and holding the felt sense, and the sense of a shift within it.
- **The inner edge** — The boundary between the felt sense and the conceptual, and the term for the felt sense itself as the place of the live process.
- **The shift** — The experience of a change in the felt sense, which is the process element of the experiential process.
- **The direction of the felt sense** — The sense of a movement or orientation within the felt sense; a blocking of which is a common finding and an important clinical target.
- **Triple structure** — Gendlin's description of the felt sense as comprising a bodily location, a sense of a whole, and a sense of a direction.
- **Six principles of focusing** — The distinctions between the felt sense and the content of the problem, bodily location, wordlessness, the shift as the process, bodily sense of a process and direction, and the felt sense versus the conceptual.
- **The special kind of listening** — Gendlin's description of the practitioner's attention: warm, wide, attending to the whole and to the wordless, and free of interpretation.
- **Patient waiting** — The practitioner's stance during a focusing session, in which the shift is allowed to occur without intervention.
- **Low-resourced focusing** — A single-session, practitioner-guided form of focusing designed for settings with limited time and resources; the variant most usable in schools and brief services.
- **Intensive focusing** — A form in which the practitioner guides the process more actively, for people who have difficulty locating the felt sense unaided.
- **Focusing as a practice** — Teaching the young person the method for their own use; the most durable form and the one most consistent with the theory.
- **The felt sense as a locus of change** — The theory's central claim: that change occurs in the felt sense, and that the felt sense is where the person has access to their own resources.
- **The person as a process** — The account of the self as a process rather than a structure; Gendlin's version converges with the contemporary person-centred formulation.

**Key Terms at a Glance (continued)**

- **Somatic symptom as held meaning** — The focusing account of a physical symptom as the localised felt sense of an unresolved situation.
- **Focusing in the body** — The technique of the body scan and the sentence stems used to locate the difficulty bodily.

## 13. Chapter Summary

Eugene Gendlin's discovery of the felt sense was made in 1946 in the course of a psychotherapy research project, and the finding was not accepted by the research team. The subsequent two decades established the phenomenon experimentally and developed the theory of experiencing.

The felt sense is a living, bodily, whole, and largely undefinable experience of one's current situation, distinct from emotion, from bodily sensation, and from thought. The theory's central claim is that psychological change happens in the felt sense and that the felt sense is the locus of the person's own resources for change. This places the theory at the intersection of the humanistic, somatic, phenomenological, and contemplative traditions, and it converges with the contemporary person-as-process formulations and with the somatic trauma literature.

The six principles and the five-step procedure make the theory teachable, and the low-resourced variant makes it usable in schools and brief services. The developmental fit with young people is unusually good: the method does not require verbal sophistication, it works with the body's primacy in emotional processing, it respects the young person's autonomy, and it is compatible with play-based work with younger children.

The experimental evidence for the bodily attention task is real and moderately strong, and the clinical outcome evidence for focusing as a therapy is considerably thinner. The theory's central claim is difficult to test, for the same structural reason that the Gestalt theory is difficult to test. The case of Arjun illustrates the theory's particular contribution: the focusing technique produced the entry into the material, the theory produced the formulation, and the developmental frame produced an intervention that changed the conditions of a young person's life.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Summarise how the felt sense was discovered, and explain why the research team did not accept it.
2. Define the felt sense, and explain how it differs from an emotion, from a bodily sensation, and from a thought.
3. State the theory's central claim about the locus of change, and explain the three practical consequences that follow from it.
4. Describe the triple structure of the felt sense, and explain the clinical use of the third element.
5. State the six principles of focusing, and give an adolescent example of each.
6. Describe the five steps of the focusing procedure, and state the practitioner's role in each.
7. Explain the significance of the "shift," and why the practitioner should acknowledge it when it occurs.
8. What does the theory say about somatic symptoms, and how does this relate to the medical assessment requirement?
9. State the developmental modifications of focusing for a six-year-old, an eleven-year-old, and a sixteen-year-old.
10. In what ways is the practice of attending to a bodily felt sense culturally marked, and what is the risk of the body's authority being assumed?

## 15. Practice Activities

**Practice and Application**

### Activity 15.1 — The Felt Sense Practicum (90 minutes, pairs)

One student locates a bodily sensation in their own body and holds it, without describing it, for ninety seconds. The partner observes and then describes what they observed — the changes in the face, the breathing, the posture, the drift of attention. Then they swap. The exercise develops two things: the ability to hold a felt sense, and the ability to observe one in another person, both of which are required in practice.

### Activity 15.2 — Low-Resourced Focusing in a Real Setting (2 hours, individual preparation, 1 hour seminar)

Prepare and deliver a single low-resourced focusing session to a peer who presents a mild, real difficulty. Use the structured instructions exactly as they are given in the manual. Afterwards, both of you write two paragraphs: what the peer experienced, and what you experienced. The second paragraph is the one to bring to the seminar, because the practitioner's experience of the session is usually the more surprising and the more instructive.

### Activity 15.3 — The Body's Untrustworthiness (60 minutes, written, then pairs)

Consider a young person who has been told for years that their body is unreliable — that their symptoms are "all in their head," that their reactions are exaggerated, that their body has failed them. Write a half-page on what the focusing method would ask of this young person, and on what the practitioner would need to do first. Then consider a young person from a cultural context in which bodily attention is not the ordinary mode of emotional processing, and write the equivalent. The two cases are different, and the comparison is the exercise.

## 16. Annotated Reading Pathways

**For the theory.** Read Gendlin (1978) first — it is short, practical, and will make the theory usable — and then read Gendlin (1996) for the clinical application. Read the 1946 research account in secondary sources as well, because the story of the finding's rejection is a useful corrective to a profession's tendency to accept only what it has already validated.

**For the developmental application.** Read Gendlin (1990), which was written for parents and teachers and is full of material that a student can use directly with a school or a family. Then read the somatic and trauma-informed literature on grounding and resourcing, and note the concepts that have been borrowed.

**For the experimental evidence.** Read the original experimental studies and one contemporary replication or review, and read the methodological critiques. The effect is real and the interpretation is genuinely contested, and a student who can state both is in a stronger position than one who can state only the first.

**For the somatic integration.** Read the material on the window of tolerance and on stabilisation, and identify the focusing-derived concepts in it. This exercise is a good preparation for Part VI, where the somatic and trauma work is developed in detail.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Focusing-Oriented Therapy and the Felt Sense** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is gendlin's experiential method and the therapeutic

use of bodily felt meaning. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The mechanism to keep in view is how safety, attention, expression, and relational responsiveness allow experience to become available for reflection and choice. The characteristic professional risk is pressuring disclosure, treating emotional intensity as progress, or neglecting risk, family context, and practical action. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in individual sessions where trust, voice, and self-understanding are central, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Amara is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Amara's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Amara explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The first intervention is deliberately modest. Amara chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Amara has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Amara can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Focusing-Oriented Therapy and the Felt Sense

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Focusing-Oriented Therapy and the Felt Sense** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Focusing-Oriented Therapy and the Felt Sense** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

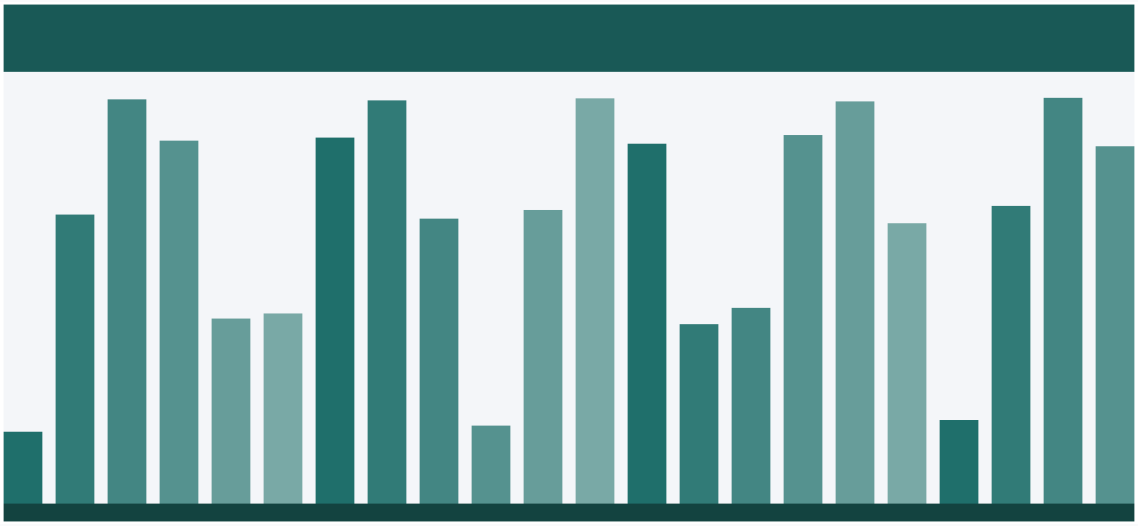
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Focusing-Oriented Therapy and the Felt Sense**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Focusing-Oriented Therapy and the Felt Sense to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 14

# Transactional Analysis: Structure, Strategy, and Script

Berne's ego-state model, the drama triangle, and the adapted use of TA with young people.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the origin of transactional analysis in Eric Berne's work with group therapy, and explain the four school divisions that followed.
- Explain the three levels of the personality model — Parent, Adult, and Child — and the characteristics of each ego state.
- Explain the concepts of transaction, transactional analysis, and script, and give a clinical example of each.
- Describe the three-level model of the person as it applies to structural analysis, strategic analysis, and script analysis.
- Apply the concepts of script, script matrix, and racket system to a case.
- Explain the concept of redecision and the redecision process, and state its clinical value.
- Apply transactional analysis to work with children and adolescents, and identify the developmental modifications required.
- Evaluate the evidence for transactional analysis, including the controversy over its scientific status.

## Chapter Overview

Transactional analysis was developed in the 1950s by Eric Berne, a physician and psychoanalyst who had trained in Vienna and who worked with group patients, many of them soldiers. Berne's founding observation was that his patients' accounts of their lives had a repetitive and dramatic structure, and that the structure could be described in a vocabulary of social roles, of exchanges, and of a life story. He published *Transactional Analysis in Psychotherapy* in 1964 and the books that popularised the field, and the theory then split into four schools — the classical, redecisional, cathexis, and integrative — each of which has a different relationship to the underlying theory and a different evidential status.

The theory is attractive to a student because it is legible. Its vocabulary is concrete, its diagrams are clear, and it offers an account of a young person's difficulty that is easy to formulate and easy to discuss. It is attractive in a second, more troubling way, and a student should be able to name it: the theory's language of Parent, Adult, and Child maps uncomfortably well onto the categories through which adults routinely think about young people, and a naive application of transactional analysis to an adolescent can reproduce the very adultism it purports to criticise. The most common error in the clinical use of this theory with young people is the treatment of the adolescent's behaviour as a "Child" state to be corrected, and the therapeutic framework in which the adult is the "Adult" doing the correcting.

The chapter presents the theory in its historical context, works through the three-level model, the concept of script, and the redecision process, and then gives the developmental and ethical attention that this theory particularly requires.

## 1. Historical Context

### 1.1 Berne and the group

Eric Berne's career is unusual. He trained as a psychiatrist in Vienna and came to the United States in the 1930s, and he worked in a variety of institutional settings, including a veterans' hospital where, in the early 1950s, he began to run group therapy with what he called "the blackout" — the group members' apparent forgetting of what had happened between sessions, which he attributed to a dissociation common to the group.

The observation that the "blackout" was really a failure to integrate the material from one session into the next, and that this was best understood as a "double layer" of the personality, was the founding insight. The social behaviour layer, Berne claimed, operated on a different level from the intrapsychic one, and each was observable in a person's speech. The "Parent", "Adult", and "Child" states were named in *Transactions in Group* (1955), and the theory of games followed in 1964.

## 1.2 The four schools

The split into schools is not merely a matter of emphasis; the schools have different theories and different statuses.

**The classical school**, Berne and his followers including Eric Berne's own formulations, Doyle and Weisz, and Harris. It retains the structural model of the three ego states, the analysis of transactions and games, and the script concept, and it tends toward a developmental model in which the Child ego state is the location of the script and the Adult is the location of the cure. The classical school's characteristic intervention is the **redecision**, drawn from Berne's 1972 *What Do You Say After You Say Hello?*, in which the client is helped to make a new decision in the area in which the old script was written.

**The redecisional school**, associated with Mary Lee and Eric Berne, and with the Ego State Institute, developed the redecision process into a systematic and directive method, with a specific protocol and a specific set of interventions. It is the most operationally specified variant and is the one most used in clinical practice.

**The cathexis school**, associated with Robert M. Young, Douglas Mosley, and Erskine, and with the Journal of theTransactional Analysis Association in its earlier form, treats the script as a set of emotional energies and payoffs, and its clinical work is with the emotional core of the script rather than with the decision. The cathexis school's contribution is the most substantial contemporary addition to the theory, and it is the school that has engaged most seriously with the attachment and the relational literature.

**The integrative school**, associated with James R. Allen, Leslie Fehmi, and the group around the International Transactional Analysis Association in its more recent form, integrates transactional analysis with attachment theory, with psychodynamic and relational theory, and with contemporary developmental and systemic thinking. The integrative school is where the theory currently has the most vitality and the most relevance to work with young people.

## 1.3 The popularity and the reputation

Transactional analysis is unusually popular outside the profession, and this has had a double effect. It has given the theory a public visibility that most clinical theories do not have, and it has produced a diffuse and sometimes quite inaccurate popular version in which "Adult" is a synonym for "healthy" and "Child" is a synonym for "spoiled." A student who has only met the popular version will be poorly prepared for the actual theory, and a student who uses the popular version will do harm.

# 2. The Three-Level Model

## 2.1 Ego states

The personality is described as organised into three ego states, each of which is a set of feelings, thoughts, and behaviours that are phenomenologically distinct and that are, in principle, available to awareness.

**The Parent.** The Parent ego state is an internalised representation of the parents or other early authority figures, and it operates by injunction and by permission. It provides a set of prohibitions ("you must not") and a set of encouragements ("you may"), and it is the source of the internal voice that judges the person. Crucially, the Parent is not the same as the actual parents, and the internalised Parent is frequently a composite of many authorities and is frequently out of date.

**The Adult.** The Adult is the component that observes, gathers data, organises it, and estimates the likelihood of the outcomes of various courses of action. It operates on the principle of reality, and it is the part of the personality that is neither feeling the feelings of the Child nor accepting the dictates of the Parent. Berne described the Adult as a data-processing function, and a student should note the striking similarity between this description and the information-processing account of the cognitive therapies.

**The Child.** The Child is the part of the person that feels, remembers, wants, and acts from the feelings. It has two divisions: the **Natural Child**, which is the healthy, spontaneous, joyful, and creative part of the young person, and the **Adapted Child**, which is the part that has learned to respond to the world in ways that are inappropriate, and which is where the hurts, the fears, the anger, and the compliance are stored.

The distinction between the Natural and the Adapted Child is the feature that most distinguishes transactional analysis from the popular version, and it is also the feature that makes it most useful with young people, because it locates the young person's vitality and their difficulties in the same place, rather than treating all of the Child as the problem.

## 2.2 The three schools of the person

The structural analysis of the three ego states yields three ways of analysing a person.

**Structural analysis** describes the composition of the personality: which ego states are present, which are excluded, and which are contaminated by which. A person whose Adult is contaminated by the Parent is a person who reasons in order to justify a position already taken; a person whose Adult is contaminated by the Child is a person who reasons from a need. A person whose Parent is excluded is a person with no internal standard, and a person whose Child is excluded is a person who is markedly inhibited and "proper."

**Strategic analysis** describes the person's characteristic way of operating: the recurrent patterns of behaviour in which the person organises their transactions with other people. It is concerned with the *how* rather than the *what*.

**Script analysis** describes the person's life story: the unconscious, preconscious, and conscious plans that the person has been living out, which were formed early, which are reinforced by parental injunctions and permissions, and which are played out over a lifetime, often with a predictable and self-defeating conclusion.

## 2.3 Transaction and analysis

A **transaction** is the exchange of a stimulus and a response between two ego states, in which the stimulus has a vector: social, psychological, or operational. A **transactional analysis** is the identification of the ego states involved in a transaction, and the analysis of the person's characteristic pattern of transactions.

The three types of transaction are:

**Complementary.** The response is at the same level as the stimulus and is of the same type, and the exchange is satisfactory. Two Adults exchanging information; a Parent caring for a Child.

**Crossed.** The response comes from a different level than the stimulus, and the exchange is confused and unsatisfying. A Parent giving an instruction to an Adult who responds as a Child; an Adult asking a question that is answered by a Parent's judgement.

**Ultraparallel.** The response is at the same level and of the same type but addresses a different topic, and the two people appear to be having a satisfactory exchange that is in fact taking place on a different subject. The clinical significance of the ultraparallel is that it is the most common form of covert communication in families, and it is a major mechanism in the maintenance of family patterns.

### 2.4 Games and scripts

A **game** is a recurring, structured pattern of exchange with a predictable payoff, which is not consciously planned and which is not in the person's best interests. Berne's account of games in general is now largely superseded by the more rigorous and more useful account of **script** that the cathexis and integrative schools have developed, and a student should learn the script account and treat the games material as historical.

A **script** is a person's unconscious life plan, formed in early childhood, reinforced by parental injunctions and permissions, and expressed in a predictable set of behaviours with a predictable conclusion. The script is not simply a set of symptoms; it is the organising story of the person's life, and the therapeutic task is to change the ending.

### 2.5 The script matrix and the racket system

The **script matrix** is the account of the three levels of the script matrix system: the underlying beliefs or injunctions (the "don't" and the "be"), the corresponding script beliefs or positions, and the corresponding behaviours. The **racket analysis** is the account of a person's characteristic "racket" — the set of covert, illegitimate, and unhealthy means by which the person seeks to get what the script promises, as distinct from the legitimate means by which the same goal might be achieved. The distinction is useful: the goal may be legitimate and the means may not be, and the means are what the therapy addresses.

### 2.6 The three areas of the script matrix

The script matrix is a three-by-three grid in which the rows are the levels of the script system and the columns are the three areas of the person's life: what the person thinks, what the person does, and what the person feels. The matrix provides a systematic way of mapping a case: what the person believes about themselves and others, what they do about it, and what they feel about it.

## 3. Script, and Its Developmental Form

### 3.1 The origins of script

Script is formed in the first years of life, in the period when the child is dependent, and it is formed out of two things: the child's own early adaptations to the environment, and the parental injunctions and permissions, which are themselves usually responses to the child's behaviour rather than independent statements. A child who is met with a consistent "you may" develops a script of permission; a child who is met with a consistent "you must not" develops a script of prohibition. Most children develop a mix, and the "mixed script" is the most common and the most difficult to work with.

### 3.2 The script and the parent's script

The developmental account of script that has emerged from the integrative school is that **the child's script is an adaptation to the parent's script**, and that the child's difficulties are frequently the result of the parent's unresolved issues rather than the child's. A parent whose script includes "I must be needed" will raise a child who is needed, and the child's symptom will be the solution to the parent's script. This account is extremely consequential for practice with young people, and it is the aspect of transactional analysis most consistent with the systemic and relational traditions.

### 3.3 The script payoff and the script belief

The modern formulation of the script has three elements, and they should be distinguished carefully. The **script belief** is the unconscious conviction about the world and the self, formed in early childhood and not available to conscious examination. The **script payoff** is the benefit, however small or however costly, that the script is designed to produce. The **script system** is the network of beliefs and behaviours that sustains it. The therapist's work is to locate the payoff — because the payoff is what the script is for — and to make it available to the young person in a form they can consider.

## 4. Redecision and the Redecision Process

### 4.1 The redecision

The redecision is the therapeutic intervention at the heart of the redecisional school, and it is the theory's most distinctive contribution. A redecision is a change in the decision, belief, or behaviour by which the person learned to survive, made now, by an adult, in the full knowledge of what the old decision purchased and what it costs.

Berne's original formulation asked the client to construct and to state a new decision in a specified area of their life, and the redecisional school has developed the protocol in detail. The protocol has six steps.

**Step 1: Establish the script belief.** The client is helped to identify the old decision, in the form "I decided that..." or "I concluded that..." — for example, "I decided that I was not the kind of person who gets what they want."

**Step 2: Trace the script belief to its origins.** The origins are located, characteristically in early childhood, and the point is to establish that the decision was a rational response to a situation in which the young person had very little choice. This step is critical: the redecision is not a criticism of the old decision, which was an intelligent adaptation.

**Step 3: Identify the payoff.** What did the old decision buy? A redecision that is not connected to its payoff is unlikely to hold, and the identification of the payoff is often the most revealing part of the work. "I

decided not to try because when I tried, I was humiliated in front of everyone, and I decided the humiliation was worse than the not-trying."

**Step 4: Identify the script belief about the self.** The decision is traced to a belief about who the person is, and this is the belief that the redecision must address. "If I don't try, then I am safe — and I am the one who never puts themselves in a position to be rejected."

**Step 5: Re-decide.** The client is invited to make a new decision, and the classical school's formulation is that the redecision is *mindful* — that is, it is made now, in the present, by an adult who has options the child did not have.

**Step 6: Test the redecision in action.** The new decision is tested in behaviour, and the testing is the point at which the old script is reasserted and has to be reworked.

### 4.2 The value of the redecision

The redecision is a clear, teachable, and time-limited intervention with a specific structure, and it maps reasonably onto the behavioural activation and cognitive restructuring work in other traditions. Its distinctive contribution is the emphasis on the *payoff*: the therapist's question is not "is this belief true?" but "what does this belief buy you, and what does it cost?" This is an unusually well-developed therapeutic stance, and it works very well with young people, for whom the question "what does this get you?" is often considerably more useful than the question "do you believe this?"

### 4.3 The cautions

The redecision protocol requires care in three respects. The step in which the origins of the decision are traced can become a re-traumatisation if the practitioner is not competent with the material, and the tracing should be done at a pace the young person can tolerate. The formulation of a new decision can become coercive if the practitioner has a particular decision in mind, and the classical school's own literature on this is extensive and worth reading. And a redecision made in the consulting room may be made to please the practitioner, and the testing step is where the practitioner finds out.

## 5. Transactional Analysis with Children and Adolescents

### 5.1 The developmental fit

Transactional analysis offers three genuine developmental contributions.

**It provides a language for the young person's experience that is not pathologising.** The concepts of the Adapted Child and the script allow a young person's difficult behaviour to be described as an adaptation with a history rather than as a character defect, and a young person who has been called defiant for six years will recognise the description of an Adapted Child differently.

**The script account locates the difficulty in the early relationship.** The developmental literature is unambiguous that early relationships are formative, and the script account provides a way of working with a young person that is developmentally congruent and that is not dependent on the young person's verbal capacity.

**The account of the Parent and the child's internalised Parent is directly relevant to the adolescent's developmental task.** The adolescent's difficulty in becoming a separate individual is a difficulty in the

negotiation of the internalised Parent, and the theory's account of the injunction and the permission is a way of working with that.

### 5.2 The specific danger

The specific danger of transactional analysis with young people is that the framework reproduces the adult's view of the young person.

The technical vocabulary of the theory — the Child ego state, the Adapted Child, the Natural Child — maps onto the ordinary adult discourse about young people, and the popular version of the theory reinforces the mapping: children are Childish, they need to be moved into the Adult, and the Adult is the person who knows. A practitioner who adopts the framework without the critical awareness will find that it validates the very attitude to the young person that the young person came to therapy to escape. The chapter's central clinical warning is this, and it should be stated in the seminar:

**In work with a young person, the Practitioner is not the Adult.** The Practitioner is one more person in a field, with their own Parent and Child states, with their own script, and with their own history. The young person's "Child" state is not the problem; it is the most alive part of them. The work is not to move the young person into the Adult but to find the Young Person's own decision, which is not the practitioner's decision and cannot be.

This is a real and specific issue, and the integrative school's development of the theory is largely an effort to address it. Erskine's work on the transactional analysis of the relationship and on the therapist's own redecision, and the integrative school's emphasis on the mutual influence of the practitioner and the young person in the here and now, are the direct responses to it.

### 5.3 The developmental modifications

**With young children**, the work is conducted through play and through the identification of the injunction, and the young child's "script" is usually observable in the repetitive play themes.

**With adolescents**, the redecision protocol is usable and is often the most direct of the interventions in this book for a young person who is stuck. The developmental modification is that the redecision must be *the young person's* and not the practitioner's, and the practitioner must be explicit about this.

**With young people in crisis**, the redecision is contraindicated in the acute phase, and the developmental and ethical judgment is that a young person in crisis has no available bandwidth for a redecision, and that the redecision can be experienced in crisis as a demand to perform a recovery that is not real.

## 6. Evidence and Research Support

**The scientific status.** Transactional analysis has had a contested standing in the scientific community since the 1980s, and the criticism has two distinct components that should be distinguished.

The first is the claim that the concepts of Parent, Adult, and Child do not correspond to anything that can be objectively measured, and that the theory is therefore not scientific. The counter-argument is that the concept of an ego state is a phenomenological description of a state of mind, and that the theory's hypotheses are about the *behavioural* consequences of the state, which is observable, and that the theory's research tradition, particularly in the integrative school, has generated testable propositions about the relationship between script characteristics and outcomes.

The second is a specific and well-documented controversy concerning the validation of the Transactional Analysis Personality Scale and the Tulley standards for the recognition of a theory of psychotherapy, and the award of the Transactional Analysis Research and Training Center's recognition to the theory in the 1990s. The dispute concerns the adequacy of the psychometric properties of the instruments and the adequacy of the process of review, and it has been conducted largely through the journals of the professional association. A student should know that the dispute exists, should read the primary sources rather than a summary of the dispute, and should not treat the controversy as evidence about the clinical usefulness of the method.

**The outcome evidence.** The outcome literature is small and mixed. A number of controlled studies, mostly in the 1980s and 1990s, have reported favourable outcomes for transactional analysis, particularly in relation to the specific outcome measures that the theory generates — script-related constructs, decisional change, and the redecision — and a smaller number have reported outcomes comparable to other bona fide therapies. A systematic review of the evidence for transactional analysis rated the evidence as limited. The integrative school's work, which is the most methodologically careful, has produced a more substantial body of process and outcome research than the classical school's, and the student who is evaluating the theory should begin there.

**The "proof" of the redecision.** The theory's own claim is that the redecision is the mechanism of change, and there is a body of research that reports associations between the completion of a redecision and favourable outcomes. The methodological difficulties in this literature are substantial: the measures are largely self-report, the redecision is assessed in ways that overlap heavily with the outcome measures, and the samples are small. The honest statement is that the redecision is a coherent intervention with a plausible mechanism, some supporting evidence, and a much weaker evidentiary base than its popularity would suggest.

**The relational findings.** As in every other humanistic theory, the support for the theory's relational propositions comes from the general alliance literature rather than from the theory's own studies.

## Further Reading for This Chapter

- Berne, E. (1964). *Transactional analysis in psychotherapy*. Grove Press. — The primary text; the beginning of the theory and the source of the script concept.
- Berne, E. (1972). *What do you say after you say hello?* Vintage. — The book in which the redecision is introduced; short and highly readable.
- Erskine, R. G. (2014). *Contact-in-relationship: Transactional analysis for the here and now*. Transactional Analysis Publications. — The most important contemporary clinical development, and the direct answer to the problem the popular version of the theory creates.
- Allen, J. R., & Bego, C. (Eds.). (2013). *Transactional analysis: A relational perspective*. — The integrative school's account, and the best single source on the current state of the theory.
- Clarkson, M. (1992). *Transactional analysis psychotherapy: An integrated approach*. — A clinical handbook.
- Sumner, J. (2010). *Looking through the glass darkly*. — A critical account of the theory's history and its controversies, written from within the movement.

# 7. Script Systems and the Case Formulation

## 7.1 Mapping a case

The three-by-three script matrix is a practical instrument for structuring a case discussion, and the exercise of filling it in with a group of students is one of the most instructive in the course, because the disagreements are productive and are usually about the *payoff* rather than about the facts.

The three columns are: what the young person believes (about the self, about others, about the world), what the young person does (the behavioural strategies, the manipulations, the avoidances), and what the young person feels (the emotional core of the script). The three rows are: the conscious level, the preconscious level, and the unconscious level.

A worked example is worth giving, because the instrument is only useful if the student can see it applied. Take a fifteen-year-old boy, Jonty, who is described by his school as "lazy, oppositional, and always lying."

**Unconscious level, belief:** "I am not the sort of person who is any good at anything, and there is no point in finding out." **Unconscious level, behaviour:** disengages from every academic task, and has learned to be charming in the first ten minutes of a session and to say nothing after that. **Unconscious level, feeling:** shame, and a terror of being seen to be stupid.

**Preconscious level, belief:** "If I don't try, I can't fail, and then it isn't my fault." **Preconscious level, behaviour:** the disengagement, the avoidance of the tests, the preference for the work he can do well. **Preconscious level, feeling:** relief, which is the payoff.

**Conscious level, belief:** "School is pointless and my teachers have it in for me." **Conscious level, behaviour:** the refusal, the lateness, the rudeness. **Conscious level, feeling:** boredom, resentment, and a conviction that he is being treated unfairly.

**The payoff.** What does the script buy? It buys the avoidance of the humiliation that would accompany a genuine attempt. The script system is coherent: the unconscious belief generates the unconscious behaviour, which generates the relief, which reinforces the belief, and the conscious layer protects the whole system with a grievance. A practitioner who addresses the conscious layer — arguing about the teachers' fairness — is arguing with the protection, and the argument will not succeed, because the young person is not lying about the teachers; the teachers are, in fact, not always fair.

**The developmental reading.** Jonty is fifteen, and the developmental question is what this pattern was in the earlier years. The interview with his mother reveals that he was a keen reader until the age of nine, when he was moved up two years and spent a year being the youngest in a class of much older children. The script was written then.

**The redecision.** In a redecisional approach, the work would be: the old decision, "I decided that trying is more dangerous than not trying, because when I try and fail, everyone sees"; the origins, in the year he was moved up; the payoff, which is the protection from the humiliation and the relief of the avoidance; the script belief about the self, "I am the one who does not put himself in a position to be found out"; and the redecision, which the young person must make himself and which the practitioner may only make possible.

**The limitation of the redecision in this case.** A redecision made in the consulting room will not survive the first test, because the script belief is held in the body and in the habit, and because the payoffs are immediate and the gains delayed. The redecision is best understood as the beginning of the work rather than

the whole of it, and the integrative school would add the work on the relationship and the work with the mother, who has her own script — a "my son has always been lazy" script which is a Parent position and which the practitioner will need to address in the family work, because Jonty's script and his mother's are, in the developmental account, two halves of one system.

## 8. The Integrative Development and the Contemporary Position

The contemporary state of transactional analysis is best described by the integrative school, and the four features of it are worth describing.

**A relational rather than a structural account of the personality.** The integrative school has largely abandoned the idea of a person with three discrete compartments and has adopted a relational account in which the person is constituted in relationship and in which the "ego states" are best understood as *positions* or *modes* that a person takes up in interaction, and in which the significant others are part of the system. This is a considerable revision of Berne's original model and it is a much better fit with the contemporary relational and systemic literatures.

**A developmental and attachment-informed account of script.** The integrative school has drawn on the attachment literature and on the developmental literature to understand how the script is formed, and this has substantially improved the theory's work with young people, because it locates the script in the early relationship and in the actual developmental history rather than in a hypothetical past.

**A here-and-now account of the therapeutic relationship.** Erskine's work on contact-in-relationship, in which the therapist attends to the felt sense of the relationship and to the young person's momentary experience of the therapist, has given the theory a here-and-now dimension that the classical school lacked, and that has made it usable in work with young people who are hypervigilant for adult inconsistency.

**A clearer ethics.** The integrative school's attention to the power differential and to the practitioner's own states has partly addressed the danger identified in Section 5.2.

## 9. Evidence and Research Support

**The status of the scientific controversy.** Two distinct issues should be separated, because they are frequently conflated and the conflation has damaged the theory's reputation unfairly.

The first is the general scientific question: whether the ego-state model corresponds to measurable psychological structures. The classical answer was that the states are phenomenologically distinguishable and can be reliably identified from speech content, and the counter-argument has been that the model lacks a validated psychometric instrument. The contemporary position, developed by the integrative school, is that the states should be understood as *positions* in a relational system rather than as structures of the mind, and that the hypotheses of interest are behavioural and relational rather than psychometric.

The second is a specific and publicly conducted dispute concerning the psychometric properties of a particular personality instrument and the process by which a particular professional body granted a theory recognition. The dispute was conducted in the journals of the professional association, and a student who wishes to understand it should read the primary sources on both sides. The general professional significance of the dispute is that it raised, and continues to raise, the question of what the profession's bodies ought to

require before they confer the status of a recognised theory of psychotherapy.

**The outcome evidence.** A small number of controlled outcome studies, mostly from the 1980s and 1990s, have reported favourable outcomes for transactional analysis in general, and a smaller number of studies have reported outcomes comparable to other bona fide therapies. The redecisional school has produced a body of process research linking the completion of a redecision to favourable outcomes, and the integrative school has produced a more methodologically careful body of work on the relationship and on script-related change. A systematic review rated the overall evidence base as limited, with small samples, weak designs, and a heavy reliance on self-report and on measures closely tied to the theory's own constructs.

**What the evidence supports.** That the redecision is a coherent, well-specified, time-limited intervention that a practitioner can learn to deliver competently, and that it is among the more time-efficient interventions in this book. That the script account provides a coherent developmental formulation for young people whose difficulties are best understood as adaptive responses to earlier circumstances. That the theory's relational propositions are consistent with the broader alliance literature.

**What the evidence does not support.** Any claim that transactional analysis is superior to other bona fide therapies for any condition. Any claim that the redecision is the unique mechanism of change. Any claim that the popular version of the theory — in which the Adult is healthy and the Child is pathological — is a valid account of a young person.

**The developmental evidence.** The specific evidence for transactional analysis with children and adolescents is thin. The developmental applications are widely described in the clinical literature and poorly studied, and the cautions set out in Section 5.2 are not reflected in the outcome literature.

## 10. Strengths and Limitations of Transactional Analysis

**Strengths.** It offers a clear and legible theory that a young person can be helped to understand and to use, and the legibility is a genuine clinical asset, particularly with young people who have been told that they are "a mess" and who benefit from a framework in which their difficulties are structured and explicable. The script account is developmentally congruent, in that it locates the difficulty in the early relationship and treats it as an adaptation with a history. The redecision is a well-specified, efficient intervention with a clear mechanism and a clear protocol. The account of the injunction and the permission is a distinctive and clinically useful way of working with an adolescent's internalised authority. And the theory's insistence on the practitioner's own states and on the here-and-now relationship has been taken up productively by the integrative school.

**Limitations.** The popular version of the theory is actively harmful when applied to young people, because it reproduces the adult's view of the child. The evidence base is thin and heavily dependent on measures closely tied to the theory's own constructs. The theory's classification of the young person's "Child" state as the locus of the problem will, in a naive practitioner's hands, become the mechanism of a new pathologising. And the theory's account of the person is individualistic, with the developmental and relational revisions of the integrative school being genuinely new rather than a matter of emphasis.

## 11. Cultural and Ethical Considerations

**The cultural location of the Parent–Adult–Child model.** The model is a product of a particular mid-century American understanding of the family: a nuclear household, with an internalised authority

figure, a reasoning self, and a natural versus adapted division of the child's experience. In cultural contexts in which the family is structured differently, in which the authority is distributed, and in which the "reasoning self" is understood as relational rather than as a separate faculty, the model's developmental assumptions do not hold. A young person from a family in which the grandmother is the primary authority has a Parent whose content is not the parents', and a practitioner who works with the model as though it were the parents' will misunderstand the case.

**The danger of the Child/Adult hierarchy.** The ethical danger is the sharpest in this book. The framework, in its popular form, holds that the Child state is immature and the Adult state is mature, and that the therapeutic task is to strengthen the Adult and to quiet the Child. Applied to a young person, this is a prescription for the suppression of exactly the parts of the young person that are healthy and vital, and it is a prescription that will appeal to adults who find the young person's affectiveness difficult. The ethical requirement is that the practitioner hold the Natural Child — the joy, the spontaneity, the play, the creativity, the capacity for feeling — as the part of the young person that is doing the most good, and that the therapeutic task is the integration of the Adapted Child's pain with the Natural Child's vitality, not the elimination of either.

**The ethics of the redecision.** The redecision raises the question of whether the young person has genuinely chosen or has been led. The ethical requirements are: the decision must be the young person's; the practitioner must not have a preferred decision in mind; the young person must be able to decline to redecide; and the practitioner must be able to work with a young person who has, in fact, decided that they do not want to change. The last of these is the one most often failed, and the failure is usually a failure of the practitioner's own anxiety rather than of their technique.

**The ethical requirement of relational safety.** The integrative school's emphasis on the here-and-now relationship and on the practitioner's own states has an ethical dimension that is easily stated. With young people, a practitioner who is reactive, who is inconsistent, or who has an unresolved script of their own will damage the work regardless of the technical quality of the redecision. The requirement is that the practitioner attend to their own reactions as data about the young person's effect on them, and to bring that to supervision.

## 12. Key Terms

### Key Terms at a Glance

- **Ego state** — A phenomenologically distinct set of feelings, thoughts, and behaviours, available to awareness; the Parent, the Adult, and the Child.
- **Parent ego state** — The internalised representation of early authority figures, operating by injunction and by permission.
- **Adult ego state** — The component that observes, organises data, and estimates outcomes, operating on the principle of reality.
- **Child ego state** — The component that feels, remembers, wants, and acts from feeling; divided into the Natural Child and the Adapted Child.
- **Natural Child** — The spontaneous, joyful, and creative part of the young person; the part most often mistaken for the problem.
- **Adapted Child** — The part of the young person that has learned responses to the world that no longer fit; the location of the hurts, fears, and compliance.
- **Structural analysis** — The account of the composition of the personality and of the contamination between ego states.
- **Strategic analysis** — The account of the person's characteristic recurrent patterns of behaviour in relation to others.
- **Script analysis** — The account of the unconscious life plan and the early decision on which it is based.
- **Transaction** — The exchange of a stimulus and a response between two ego states, having a social, psychological, or operational vector.
- **Complementary transaction** — A mutually satisfying exchange between equivalent ego states.
- **Crossed transaction** — An exchange in which the response comes from a different ego state than the stimulus, producing confusion.
- **Ultraparallel transaction** — An exchange in which both parties address different subjects while appearing to communicate; a major mechanism of covert family communication.
- **Game** — A recurring patterned exchange with a predictable and self-defeating payoff; largely superseded by the script concept.
- **Script** — An unconscious life plan formed in early childhood, reinforced by parental injunctions and permissions, and expressed in predictable behaviours.
- **Script belief** — The unconscious conviction about the world and the self that underlies the script.
- **Script payoff** — The benefit, however small or costly, that the script is designed to produce; the key to the therapeutic work.

**Key Terms at a Glance (continued)**

- **Script matrix** — A three-by-three grid mapping the levels of the script system against the areas of belief, behaviour, and feeling.
- **Racket system** — The covert, illegitimate means by which a person seeks to obtain what the script promises, as distinct from the legitimate means.
- **Injunction** — A parental prohibition or command that becomes part of the young person's internal standard.
- **Permission** — A parental allowance that becomes part of the young person's internal standard.
- **Redecision** — A mindful change made now, by an adult, in a decision, belief, or behaviour originally made in childhood.
- **Redecision process** — The six-step protocol: establish the script belief, trace its origins, identify the payoff, identify the script belief about the self, redecide, and test in action.
- **Contact-in-relationship** — Erskine's account of the therapist's attention to the here-and-now experience of the relationship with the young person.
- **Script system** — The network of beliefs, behaviours, and payoffs that sustains a script.

## 13. Chapter Summary

Transactional analysis developed from Berne's observation, made in group therapy with soldiers in the early 1950s, that his patients' accounts of their lives had a repetitive and dramatic structure that could be described in a vocabulary of social roles, exchanges, and life stories. It split into four schools — classical, redecisional, cathexis, and integrative — with different theories and different evidential standings.

The three-level model describes the Parent, the Adult, and the Child, with the Child's division into the Natural and the Adapted being the feature that most distinguishes the theory from its popular version. Structural, strategic, and script analyses describe the person's composition, their characteristic patterns, and their life plan. The script concept has been refined into the modern account of a script belief, a script payoff, and a script system, and the redecision is the theory's most distinctive intervention, with a six-step protocol that identifies the old decision, traces its origins, establishes its payoff, and invites a new one.

The developmental fit with young people is genuinely good in the script account, which locates the difficulty in the early relationship and treats it as an adaptation with a history. The specific danger is equally clear: the framework's hierarchy of Child and Adult reproduces the adult's view of the young person, and a practitioner who adopts the framework naively will validate the very attitude the young person came to escape. The requirement that the practitioner hold the Natural Child as the part doing the most good, and that the redecision be the young person's own, is the ethical core of the theory's contemporary development.

The evidence is limited and heavily dependent on measures closely tied to the theory's own constructs. The controversy over the theory's scientific recognition was conducted in the professional journals and should be read in the primary sources. The case of Jonty illustrates the script matrix in use and shows how a redecision, in a case with an early and painful origin, must be tested in action rather than made in the session.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe Berne's founding observation and explain why the "blackout" was significant.
2. Distinguish the four schools of transactional analysis and state the clinical difference between the classical and the integrative.
3. State the three ego states and the six primary features of each.
4. Explain the difference between the Natural Child and the Adapted Child, and why the distinction matters in work with young people.
5. Distinguish structural, strategic, and script analysis, with an example of each.
6. What is a crossed transaction and what is an ultraparallel transaction, and why is the latter clinically important in family work?
7. State the six steps of the redecision process, and explain why the identification of the payoff is the most important of them.
8. Explain the developmental account in which the child's script is an adaptation to the parent's script, and state its clinical consequence.
9. State the specific danger of the transactional analysis framework in work with young people, and describe the clinician's own state in that session.
10. In the Jonty case, why would a practitioner who argued about the teachers' unfairness fail, and what would the argument need to address instead?

## 15. Practice Activities

**Practice and Application**

### Activity 15.1 — The Script Matrix Workshop (2 hours, groups of four)

Each group takes a case and fills in the three-by-three script matrix in the group, marking each cell with the group's degree of confidence. The groups then compare matrices, and the comparison is the exercise: the groups almost always agree on the factual material and disagree on the payoffs, and the disagreement is usually the most useful part of the session. The group should then produce a single sentence stating the young person's early decision.

### Activity 15.2 — The Redecision Protocol Rehearsal (90 minutes, triads)

One student presents a real or imagined stuck point, one conducts the six-step protocol with the triads' advice, and one observes for the point at which the "new decision" became the practitioner's decision rather than the client's. The observer's task is to mark the exact sentence, and the practitioner then responds to it in the next session. This is the most practically useful exercise in the chapter and it is also the most demanding, because the pressure to supply the young person with a good decision is strong.

### Activity 15.3 — The Popular Version Audit (60 minutes, written)

Write a 500-word account of transactional analysis as it is usually presented to the public — on a popular psychology podcast, in a self-help book, in a training advertisement. Then write a second 500-word account of it as the integrative school would present it. Then list the differences, and identify which of the differences are matters of clinical fact and which are matters of framing. The exercise develops the critical eye that the theory requires, and it is best done before the student begins using any of the concepts with a real young person.

## 16. Annotated Reading Pathways

**For the theory.** Read Berne (1964) for the structure and the script, and Berne (1972) for the redecision. Read them in that order, and read them with a pencil.

**For the contemporary clinical development.** Read Erskine (2014) and the integrative school's edited volume. This is where the theory is strongest and where the practical guidance for work with young people is clearest, and it is very little read.

**For the controversies.** Read the primary sources on both sides of the scientific recognition dispute, in the professional journals. The dispute is a useful case study in how a profession handles an internal disagreement, and the student should be able to describe it accurately and without taking a partisan position.

**For the development.** Read the attachment and developmental literature that the integrative school has drawn on, and read the developmental applications in the clinical literature with attention to the cautions in this chapter rather than to the enthusiasm.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Transactional Analysis: Structure, Strategy, and Script** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is berne's ego-state model, the drama triangle, and the adapted use of TA with young people. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather

than merely continuing. The organising focus for this part of the course is awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The mechanism to keep in view is how safety, attention, expression, and relational responsiveness allow experience to become available for reflection and choice. The characteristic professional risk is pressuring disclosure, treating emotional intensity as progress, or neglecting risk, family context, and practical action. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in individual sessions where trust, voice, and self-understanding are central, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Jonas is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Jonas's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Jonas explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The first intervention is deliberately modest. Jonas chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Jonas has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Jonas can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Transactional Analysis: Structure, Strategy, and Script

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Transactional Analysis: Structure, Strategy, and Script** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Transactional Analysis: Structure, Strategy, and Script** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

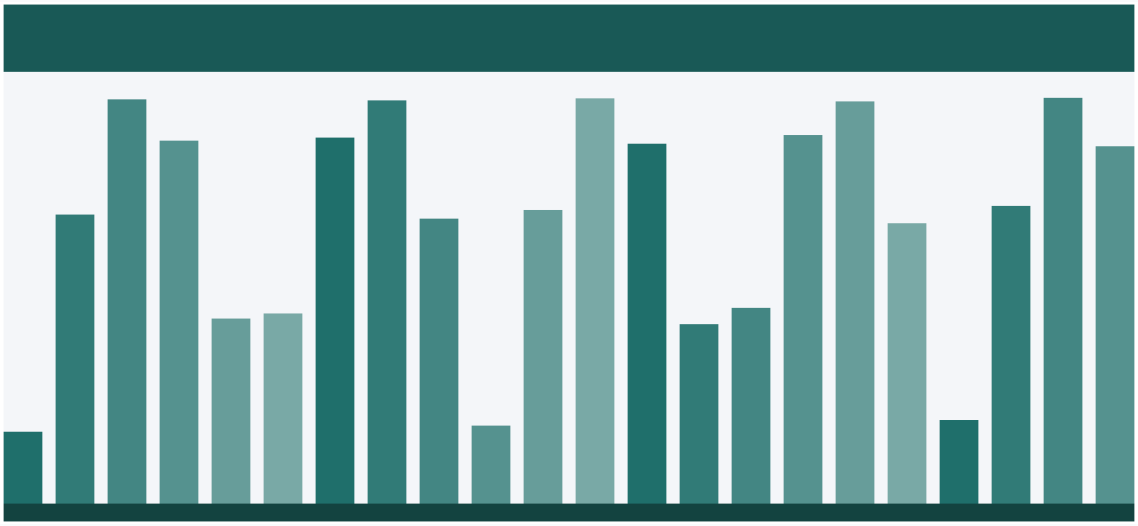
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Transactional Analysis: Structure, Strategy, and Script**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Transactional Analysis: Structure, Strategy, and Script to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 15

# Internal Family Systems and Structural Dissociation

Schwarz's model of parts and the treatment of partitioned experience in traumatized youth.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the development of internal family systems therapy and the influence of structural dissociation theory on it.
- State the three levels of experience in the internal family systems model and distinguish the Self from the parts.
- Describe the five categories of internal parts — Exiles, Managers, Firefighters, Self, and the ubiquitous Outsiders — and the function of each.
- Explain the concepts of blending, unblending, the 8 Cs of Self, and their clinical use.
- Describe the six-step therapeutic process and the role of the practitioner in it.
- Apply internal family systems thinking to a case involving a young person, and specify the developmental modifications required.
- Evaluate the evidence for internal family systems therapy, and state the current status of the randomised controlled trial evidence.
- Explain the ethical requirements that the model generates, particularly with respect to the young person's autonomy and to the treatment of difficult behaviours.

## Chapter Overview

Internal family systems therapy was developed by Richard Schwartz in the 1980s, initially as a way of working with the eating disorders and substance abuse that Schwartz encountered in his clinical work, and it has since developed into one of the most widely used integrative therapies in the field. Its central claim is that the mind is naturally multiple — that each of us is made up of a number of distinct subpersonalities, or parts, each with its own perspective, its own strategy, and its own history — and that the symptom is the result of a conflict among the parts rather than a failure of a single unified self.

The theory is unusual in the profession in that it is explicitly and deliberately religious in its origins and has subsequently developed a secular, non-spiritual version for clinical use, and in that the two versions have been somewhat confusingly run together in the literature. The model described in this chapter is the secular clinical model, and a student should be aware that the "Self" in the model is a specific clinical construct with a specified set of attributes and should not be confused with any metaphysical entity. Chapter 48 returns to this in the context of the profession's general avoidance of metaphysical claims.

The theory's relevance to work with young people is considerable and is not always exploited. The parts model provides a language for the young person's internal conflicts that is immediately comprehensible, that does not require the young person to be verbally sophisticated, and that locates the symptom as a solution. The developmental application is in its early stages and the evidence base for it is thin, and both of these facts should be stated by the student rather than glossed.

## 1. Historical Context

### 1.1 The clinical problem that produced the theory

Schwartz's original clinical work was with women with eating disorders and with substance abusers, and the observation that produced the theory was that these patients were not internally consistent. They were, on the contrary, in a state of conflict between powerful internal forces that were pulling in different directions, and the eating disorder and the substance abuse were, on this reading, the products of a conflict rather than the expression of a single disordered state.

This observation was strongly reminiscent of a specific body of theory that Schwartz encountered in the clinical literature, and that connection is the second origin of the model.

### 1.2 Structural dissociation

The theory of **structural dissociation**, developed principally by the Dutch trauma researchers Jan van der Hart, Antonio Onno van der Hart, Sandra Aeltsma and their colleagues, and drawing on Pierre Janet's nineteenth-century work, proposes that a person exposed to overwhelming trauma is unable to integrate the traumatic experience into the whole of the personality. What happens instead is a division of the personality into two or more relatively autonomous "action systems": the **apparently normal part**, which continues to function in the ordinary world and has no access to the traumatic memories and the associated affect, and the **emotional part**, which is fixated in the trauma, is dominated by the traumatic affect, and is temporally located in the moment of the trauma.

The theory's account of the symptom is that the two systems are in conflict, and that the symptom arises from the conflict and from the burdens carried by the emotional part. The account of the treatment is that the work is to enable the integration of the two systems, which requires establishing a relationship with the emotional part and helping it complete its unfinished action.

The theory is important to this chapter for two reasons. It provided Schwartz with a theoretical account of the internal multiplicity he was observing, and it has provided the wider field with a rigorous and increasingly influential account of the mechanism of dissociation that underlies many of the presentations that come to youth services. Chapter 38 develops the structural dissociation material in the context of trauma, and Chapter 40 develops it in the context of stabilisation.

### 1.3 The development of the model

Schwartz's initial formulation described three categories of part — Exiles, Managers, and Firefighters — and was derived from his clinical work with eating disorder and substance abuse patients. He later added the **Self** as a distinct entity with its own attributes, and the model was further developed by the addition of the category of Outsiders, by the development of the six-step process, and by the subsequent work on the treatment of families and schools, on the online delivery of the therapy, and on the treatment of psychosis.

## 2. The Three Levels of Experience

The model is described in terms of three levels of experience, and a student should be able to describe the relationship between them precisely.

**The unblended Self** is the core of the model. It is a distinct level of experience that is not a part and that has its own qualities and its own developmental trajectory. It is the level from which the practitioner works, and the goal of therapy is that the client develop more of it.

**Parts** are discrete aspects of the person's experience, each with its own perspective, its own emotional style, its own agenda, and its own history. A part is not a symptom, and it is not a disorder; it is a subpersonality with a legitimate adaptive function and a strategy that was once appropriate.

**Blending** is the condition in which a part's perspective is running the show, so that the person is *in* a part rather than *has* a part. Blending is a normal and necessary condition of daily life; the clinical problem is the degree of blending and the fact that the person has lost access to the Self while it is occurring.

### 2.1 The Self and the 8 Cs

The Self is described in terms of eight qualities, and the 8 Cs is the practical handle on it. The Self is **curious**, **calm**, **compassionate**, **confident**, **connected**, **courageous**, **clear**, and **committed**.

The 8 Cs are not aspirational virtues; they are descriptive of a state of experience that the practitioner can learn to recognise in the client and to cultivate in themselves. The Self is characterised by **emotional curiosity** — a genuine interest in the parts and in what they are protecting — by **emotional calm** — the capacity to be present with intense affect without being overwhelmed by it — and by **emotional compassion**, which is directed at the parts rather than at the behaviour. The three emotional Cs are what the practitioner works with most directly, and the working definition of the therapeutic stance in the model is "a curious, calm, and compassionate relationship with a part."

The Self is not a metaphysical entity and should not be presented as one. A secular reading of the Self is as the person's own adult, reflective, perspective-taking capacity, operating within the developmental and relational context in which it was formed — a reading that is close to the account of the self as a process in the person-centred tradition and to the account of reflective functioning in the mentalisation-based tradition discussed in Chapter 42.

## 3. The Categories of Part

### 3.1 Exiles

**Exiles** are the parts that carry the young person's wounds: the experiences of hurt, fear, shame, abandonment, and injustice that were inflicted on them and that they were not able to process at the time. Exiles are the youngest parts, and they are the parts most vulnerable to the external environment. They are often hidden, and the young person may have no conscious awareness of them until the therapeutic relationship provides sufficient safety for them to emerge.

An Exile in a young person carries the emotion of a specific past experience. A sixteen-year-old who was repeatedly humiliated at school may carry an Exile that feels like the humiliation itself, and the part may be as young as the child who experienced it, and it may have a correspondingly young vocabulary.

### 3.2 Managers

**Managers** are the parts whose function is to prevent the Exiles from being triggered, and their strategies are proactive. A Manager may be the part that plans, that organises, that controls, that achieves, that analyses, that works, that keeps the peace, that suppresses, or that withdraws. The Manager's function is protective and is aimed outward, at the prevention of the Exile's activation.

In a young person, the Manager strategies are the strategies of a child who has had to manage a difficult environment: the pleasing, the high achievement, the silence, the compliance, the caretaking of the parent, the use of humour, the withdrawal. The Manager is the part of the young person that has learned to be impressive, and the developmental cost of that is the loss of the parts that were not impressive.

### 3.3 Firefighters

**Firefighters** are the parts whose function is to extinguish the fire of the Exile's activation, and their strategies are reactive. The Firefighter may be the part that dissociates, that numbs, that bingeing or purging, that drinking or using, that raging, that suiciding, that cutting, that zoning out, that acting out, or that over-eating. A Firefighter's strategy is urgent, disproportionate, and aimed at the immediate reduction of the Exile's affect.

The distinction between a Manager and a Firefighter is a matter of timing and of direction, and it is the most useful single distinction in the model for work with young people: a Manager prevents; a Firefighter reacts. A young person who is a perfectionist at school and who bingees at night is displaying both, and the two are in a relationship in which the Manager's success generates the conditions for the Firefighter's activation.

### 3.4 Outsiders

**Outsiders** are the parts that maintain the appearance of things, and they include the part that criticises the system, the part that is the sceptic, the part that wants to make sure the therapy is right, and the part that is depressed by the work. The Outsider's function is to keep the system safe from the intensity of the work, and in clinical practice the Outsiders appear as the young person's persistent scepticism about the therapy, the part that says "this won't work", the part that intellectualises the whole thing, and the part that wants to quit.

The Outsiders are the least discussed category in the clinical literature and the most practically useful with adolescents, because the adolescent's scepticism about therapy is often a legitimate Outsider rather than a resistance, and a practitioner who treats it as resistance will fail.

### 3.5 The Symptom as the Solution

The model's account of the symptom is the one that makes it clinically powerful: **the symptom is the solution of a conflict among the parts.** The symptom is what the system produces in order to resolve the conflict, and it is a solution in the only sense that matters clinically — it works. It reduces the conflict, and the young person can function. The price is the continuing conflict and the continuing expenditure of the young person's resources, and the price is often paid in development, in relationships, and in the body.

The clinical corollary is important and is frequently mishandled: the symptom should not be attacked, and the parts that produce the symptom should not be treated as the enemy. They are protectors, they are doing their jobs, and the work is to understand what each is protecting against and to find a way for the system to be reorganised so that the protection is no longer required.

## 4. The Therapeutic Process

### 4.1 The six steps

**Step 1: Mapping.** The practitioner and the young person work out the internal world of the young person — the parts, their roles, their relationships, and the conflicts among them. The mapping is done through the young person's own language and their own images and feelings, and the practitioner does not impose a structure. The visual mapping can be used with young people, and the use of drawing is an important development in the child and adolescent applications.

**Step 2: Connecting with the Self.** The practitioner helps the young person find, and to be in, the Self, and the working is through the 8 Cs. In practice this is done by attending to curiosity, calm, and compassion, by asking questions that invite the young person's own perspective on the parts, and by noticing when the practitioner or the young person has slipped into a part.

**Step 3: Understanding the protectors.** The practitioner works with the Manager and Firefighter parts, understanding what they are protecting the Exile from, and what they believe is necessary. The work with protectors is respectful and curious, and it is the part of the process that most requires the practitioner to be free of a part's own perspective — the protective part will describe the young person as a defective or dangerous one, and the practitioner must hear that description without accepting it.

**Step 4: Healing the Exile.** When the Self is sufficiently established, the practitioner helps the young person connect with the Exile, and the Exile is allowed to bring its story, its emotions, and its memory. This is the most affectively intense step, and it is the step at which the model's account of dissociation becomes clinically critical, because in a young person with a trauma history the Exile is a part of a dissociated system, and the connection must be made with great care and at a pace the young person can tolerate.

**Step 5: Retrieving.** The Exile's burden is retrieved and the Exile is reoriented towards its original caretakers or sources of the pain. The idea is that the pain belongs to those who caused it, and the Exile can be released from it. The procedure involves allowing the Exile to give back what it was given, and to be shown, in the therapeutic relationship, a different outcome from the one it has been expecting.

**Step 6: Reworking.** The parts are reorganised so that the conflicts are resolved and the old strategies are no longer required. The young person is left with the Self, with access to the parts, and with the capacity to choose among them.

## 4.2 Unblending

The concept of **unblending** is the model's core technical idea and it is what makes the approach more than a re-description. Blending is a normal condition and the goal is not to eliminate it but to increase the young person's capacity to notice it and to step out of it. The practitioner works with the young person to develop a "distance" from each part: to experience the part's feelings and thoughts while also experiencing themselves as the one who has them.

Practically, this is done by attunement to the shifts in the young person's state, and by naming what is happening: "It sounds like the part that has to keep everything under control is here right now. Can you hear yourself saying that?" The naming gives the young person a choice that they did not have in the moment of blending, and the capacity to exercise that choice is the mechanism of change in the model.

## 4.3 The practitioner's position

The practitioner's position in the model is a specific one, and it is different from the person-centred position. The practitioner works from the Self, and the practitioner is a *real* part of the client's system, which means that the practitioner can become blended with a part of the client's system. A practitioner who is caught up

in the young person's Firefighter may become the protector who is being sought, and a practitioner who is caught up in the young person's Manager may become the achievement-oriented adult who colludes with the over-functioning. The model's contribution to the therapist's self-awareness here is real and it is one of the more practically valuable things in the theory.

# 5. Internal Family Systems with Children and Adolescents

## 5.1 The developmental fit

The developmental fit of the model with young people is good, and there are three specific reasons.

**It externalises the internal.** For a young person, the most difficult thing about their own mind is that it is a jumble and that they cannot get any distance from it. The parts model provides a structure in which the young person can have a view of their own mind from the outside, and the use of drawing and of externalised representation makes the structure available to a young person who cannot hold it verbally.

**It is non-blaming in a way that young people can hear.** The young person who has been told they are "attention-seeking" or "defiant" or "in denial" can recognise, in the model, that the part that is doing these things is a protector with a function, and the recognition is often the first moment in which the young person feels understood.

**It is compatible with the developmental reality of the self.** The model's account of the Self as a capacity that develops and that can be temporarily lost is developmentally congruent, and it maps well onto the developmental literature on the adolescent's developing reflective functioning.

## 5.2 The developmental modifications

**With younger children**, the work is conducted through play, drawing, and image, and the parts are represented as characters in the young person's own imagery. The child who has a "worried one" and a "brave one" is already doing parts work, and the practitioner joins the child's metaphor rather than translating it into adult language.

**With adolescents**, the model is used in a more explicit form, and the specific development is the use of visual mapping. The adolescent who can draw a system of parts and see the relationships among them has made a significant step, and the practitioner can then work with the drawing as the shared object.

**With young people in crisis**, the model has a specific and important contribution: the crisis is understood as a Firefighter in action, and the first task is to connect with the Self, which is the part of the young person that is not in crisis. A young person in acute crisis is in a part, and the practitioner works to find the Self that is not — which is a different and more tractable task than trying to talk to the young person out of the crisis.

## 5.3 The specific cautions

**The Exile work.** The step in which the young person connects with the Exile and tells the trauma story is the highest-risk intervention in the model with a young person, and the developmental and ethical requirements are strict: adequate stabilisation, adequate capacity to remain in contact, a young person who can consent meaningfully to a procedure they understand, a practitioner competent in the work, and a support structure that will be in place afterwards. The structural dissociation literature is explicit that

approaching the emotional part of a dissociated system without adequate stabilisation can retraumatise.

**The risk of the model being experienced as a past-life or possession framework.** A young person who is religious, or who has a spiritual framework, may hear the parts language in a way the practitioner does not intend, and the secular model should be explained carefully. Conversely, a young person who has been told by a family member that they are "possessed" or "demonic" may find the parts language a relief. The practitioner should ask rather than assume.

**The risk of over-intellectualisation.** The model is a framework for understanding, and a young person with a good intellectual grasp can learn the vocabulary of parts and use it to talk about their difficulties without ever being in contact with them. The Practitioner must watch for this, and the clue is whether the young person's affect changes in the session.

## 6. Modern Developments

The theory has developed in several directions that are relevant to a graduate student.

**Internal family systems for families.** The model has been extended to work with the whole family system, in which each family member has a set of parts and the family's symptoms are understood as the product of the interaction of the parts across the members. This is a considerably more complex application than the individual model, and it is developing rapidly.

**Internal family systems in schools.** A school-based model, "school-based internal family systems," has been developed and trialled, and it is the development most directly relevant to the student in this course, because it adapts the model to a brief, time-limited, educational setting with a young person who may have limited trust and limited continuity.

**Online internal family systems.** The delivery of the therapy online, developed in response to the pandemic, has produced a substantial research literature, and the findings are relevant to Chapter 55.

**Internal family systems for psychosis.** The model has been applied to the treatment of psychosis, and this application is significant because it is the clearest instance in the contemporary field of a therapy organised explicitly around the premise that the person's experience is not a set of symptoms to be eliminated but a set of parts in conflict, and because it engages directly with the experience of the person with psychosis rather than with the symptoms.

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## Further Reading for This Chapter

- Schwartz, R. S. (1995). *Internal family systems therapy*. Guilford. — The primary clinical text.
- Schwartz, R. S. (2004). *Internal family systems therapy: The clinical guide*. — The more detailed guide to the process, and the one to use with a client.
- Brehm, K. (2021). *Connecting with the parts: A workbook for internal family systems therapy*. — A practical workbook, useful with adolescents and families.
- van der Hart, J. M., van der Hart, A. O., & van Loon, J. (2022). *A concise introduction to structural dissociation theory*. — The clearest account of the underlying trauma theory.
- Guardini, M. (2018). *Working with internal family systems in schools*. — The school-based application.

- Jansen, J. L., & Coffey, S. F. (2019). *Internal family systems therapy for psychosis*. — The psychosis application, and an important demonstration of the model's reach.

## 7. Case Illustration: The Girl Who Could Not Say No

**The referral.** Halima, fifteen, is referred by her school for "anxiety and possible eating disorder." The school reports that Halima has lost a significant amount of weight over the past four months, that she is skipping lunch, that she is "constantly busy" with a very demanding extracurricular programme, that she is "top of the set in everything", and that she has not been eating with her family for months. Halima's mother, who brought her to the appointment, says that Halima "has always been high-achieving" and that "we are a family that does things properly", and that she is "very proud of her."

Halima is polite, articulate, and warm. She answers every question fully. She does not appear distressed. She has lost, by her own account, "no" to nothing in particular — she has simply stopped noticing the requests.

**The parts mapping.** The practitioner asks Halima to describe the internal experience of her week, and then asks a question that is simple and, for this young person, disorienting: "Who is doing all of that for you?" Halima's face changes, and after a pause she says: "I don't know what you mean." The practitioner asks: "When your mother says she's proud of you, who is pleased?" Halima says, in a slightly different voice: "She is. She is proud. That's what matters." And then, in a third voice: "But it doesn't matter, does it? That's the problem."

**The map.** The map emerges over three sessions and is drawn by Halima herself, in a notebook, at the practitioner's suggestion.

*One part is the one who is organised.* She describes this part as the one who knows what has to be done, who gets the forms completed, who gets to school on time, and who gets the grades. She says: "If I don't do it, nobody does it. My mother works, my dad is in Leeds half the week, and Abdi does not do anything." She calls it "the manager" after the practitioner explains the model.

*Another part is the one who cannot say no.* She describes it as "the one who gets asked" and she says: "It just comes out. My cousin asks me to help with her coursework and I say yes before I have finished the sentence." She calls this "the yes part."

*A third part is the one who is hungry.* Halima draws this part as very small, and in a corner, and she says little about it. The practitioner does not press.

*And there is a part that is very quiet.* Halima says, "There's one that doesn't say anything. I don't know. It's just there." The practitioner asks what it does. Halima says: "It's the one that knows."

**The formulation.** The map shows a system in which an Exile — the small, hungry, hidden part — is being protected by two protectors with quite different strategies. The Manager part organises Halima's life so tightly that the Exile is never triggered: no time is available for a hungry feeling, because the schedule is full and the achievements are constant. The "yes" part, the Firefighter, acts on the Exile's affect: it accepts the requests, the commitments, the demands, so that the Exile's distress never has to be felt directly. And the quiet part that "knows" is the young woman's own capacity that has been crowded out of the schedule, which the practitioners describe as her Self in its developmental form.

The developmental reading is crucial and the practitioner works it into the formulation explicitly: this is a fifteen-year-old who is doing a caregiving and a self-regulation job that belongs to an adult, in a household where the adults are themselves stretched, and where "we are a family that does things properly" is a Parent injunction that has been transmitted intact. Halima's weight loss is the solution to the conflict among the parts, and it is working: it is the only part of Halima's experience that is not being managed.

**The intervention.** The practitioner does not go to the Exile, and the practitioner is explicit with Halima about why: "I want to meet the part that is hungry, and I am not going to do that today, or probably next week, because the part that is organising your life is doing a very good job of keeping that part safe, and if I go there too quickly we will both be frightened."

The work proceeds at the level of the protectors. The practitioner asks Halima to spend time noticing the organising part, to notice when it is in charge, and to notice what it protects. And the practitioner asks about the "yes" part: "What happens if you say no? What does the yes part think will happen?" The answer is revealing: "It thinks everything falls apart. It thinks my dad won't come back from Leeds. It thinks my mum will have to give up the job."

**The family work.** The intervention that makes the difference is the family work, and it is the intervention the individual model would not have prompted. The practitioner asks Halima's mother to come to a session, and asks Halima first. The session is not about blame and it is not about the eating. The practitioner asks the mother: "What do you need Halima to be?" The mother is quiet, and then says: "I need her to be the one who holds it all together. I am not sure I could hold it if she didn't." And the practitioner asks Halima: "Can you hear that? What do you do with that?" And Halima says: "That is what the hungry part is about. If I am not the one who holds it all together, then who does?"

This is the moment at which the Exile becomes reachable, and the practitioner says so and does not push: "That is the part we have been looking for. We will come to it. Not yet."

**The trajectory.** The work over the following months includes: the mother engaging with what she needs her daughter to be; Halima's "yes" part being examined, with the gradual capacity to decline one request; the Manager part being examined, with the first small and deliberate inefficiency; and the eventual work with the Exile, in which the young woman is able to say what the hunger is for — the eating, the weight loss, the shrinking, the wish to be small enough to be needed by a family that is also in trouble. The outcome is not a resolution of the family system. The outcome is that Halima gains some weight, that the "yes" part is less active, and that the quiet part that knows is more present in the sessions.

**The ethical dimension.** Three requirements were engaged. The first is the requirement of medical assessment, which the practitioner insisted on at the outset: a fifteen-year-old with significant weight loss requires medical evaluation, and the therapist's framework cannot substitute for it. The second is the requirement of parental consent and of the young person's assent, and the practitioner was careful to work with Halima's mother in a way that did not covertly triangulate the mother against the daughter. The third is the requirement to protect the young person from the "both sides" technique, in which the practitioner invites the young person to differentiate from the parent's position in a way that puts the young person at risk of the parent's subsequent anger. The practitioner slowed the family work and did not proceed with the differentiation until the mother's own capacity to tolerate the differentiation had been assessed.

## 8. Evidence and Research Support

## 8.1 The outcome evidence

The evidence base for internal family systems therapy has grown substantially over the past fifteen years, and the quality of it has improved.

**The early evidence.** A number of uncontrolled outcome studies and case series from the 1990s reported favourable outcomes, and the small number of controlled studies in the 1990s and early 2000s were generally positive but small.

**The more substantial contemporary evidence.** A series of larger studies, several of them randomised controlled trials, have been conducted across a range of populations and conditions, and the results are among the more encouraging of the recent integrative therapy literature. The conditions and populations studied have included depression, anxiety and related disorders, eating disorders, substance use, chronic pain, and adolescent presentations including self-harm and eating pathology. A systematic review of the therapy in adults found a substantial and growing body of randomised controlled trial evidence with effect sizes comparable to those of established treatments, and the developmental and adolescent literature has grown, though more slowly.

**The online evidence.** The body of randomised controlled trials of online internal family systems therapy is among the largest in the field for any integrative therapy, and it includes a series of large randomised trials with good methodological quality. The findings are consistent with the effectiveness of the model and, importantly for this course, with the acceptability of the model for populations who would not otherwise present for therapy.

**The limitations.** The methodological quality of the studies is variable, and a substantial number are conducted by investigators with an allegiance to the model, which is a legitimate concern in a literature of this size. The long-term follow-up data are limited. The evidence for the family and school applications, which are the most relevant to this course, is much thinner than the evidence for the individual adult application. And the evidence for the psychosis application, which is clinically important, rests on a small number of studies.

## 8.2 What the evidence supports

That internal family systems therapy is an effective treatment for a range of conditions in adults, with effect sizes in the range typically considered moderate to large. That the model can be delivered remotely with comparable outcomes. That the model's premise — that the symptom is the solution of an internal conflict — is a workable and clinically useful frame, as evidenced by the consistency of the outcomes across the range of conditions studied. And that the six-step process is deliverable by trained practitioners with fidelity.

## 8.3 What the evidence does not support

That internal family systems therapy is superior to other treatments for any condition; the comparative evidence does not show this. That the parts model is a metaphysically accurate description of the mind. That the model can be delivered safely to young people with a trauma history without the extensive preparation the structural dissociation literature requires. And that the model can be delivered in a brief or unsupervised form; the evidence base is for the full model with trained practitioners.

## 8.4 The status of the "Self"

The Self is a clinical construct in the model and the published outcome literature measures the process of therapy and the outcome, not the Self as an entity. The Self's status in the theory's non-clinical formulation is a matter of continuing and unresolvable internal disagreement within the movement, between those who regard the Self as a spiritual or transpersonal entity and those who treat it as a secular, developmental, and relational capacity. A student should be aware of the disagreement and should adopt the secular formulation in clinical work, in which the Self is described through the 8 Cs and is understood as a developmental and relational capacity, and should not present the transpersonal formulation to a client as a scientific claim.

## 9. Strengths and Limitations of Internal Family Systems Therapy

**Strengths.** It offers an account of the symptom as a solution, which is both non-blaming and immediately actionable, and which is very often the first thing a young person needs to hear. The externalisation of the internal provides a structure that a young person can see and work with, and it is compatible with drawing, with play, and with the visual mapping that has become the standard approach with adolescents. It is non-linear and non-pathologising without being vague, and it generates concrete technical steps that a practitioner can learn. It has one of the better bodies of contemporary evidence among the integrative therapies, and it has been unusually well adapted to online delivery. The Exile work is directly informed by the structural dissociation literature, which makes the model considerably safer than it would otherwise be with trauma-exposed young people. And the model's insistence on the practitioner's own position and on noticing when the practitioner has become caught up in a part is a genuinely valuable clinical discipline.

**Limitations.** The model's subtlety is a real barrier: the vocabulary is easy to misuse, and a practitioner who uses the language of parts without the discipline will produce a shallow and often irritating performance. The model can be experienced by a young person as a sophisticated adult game. The family and school applications are poorly studied. The evidence base is heavily weighted to adult populations and to individual work. And the metaphysical dimension of the theory in its original form makes it difficult to use in a professional context in which the practitioner may not hold the metaphysical view, which is a practical problem that the secular model addresses but does not eliminate.

## 10. Cultural and Ethical Considerations

**Cultural considerations.** The model's account of the internal world is, like most of the field's, an account of an individual mind. Its applicability to young people whose psychological difficulty is substantially located in relationship, in family obligation, in community, or in structural conditions is not well developed, and the family's role as a system that includes the young person's parts is a complicated matter in cultures in which the family is the young person's primary context of selfhood. The framework's own developmental account — in which the young person's parts and the parent's parts are two halves of one system — is an interesting contribution here, and it deserves more application than it has received.

**The ethical question of the Exile work.** The model's highest-risk intervention is the work with the Exile, and the ethical requirements are exact: adequate stabilisation; a young person who can consent meaningfully to a procedure they understand; a practitioner competent in the work and in the management of intense affect; support in place for the aftermath; and a plan for what happens if the young person becomes destabilised. The structural dissociation literature is explicit that approaching the emotional part of a

dissociated system without adequate stabilisation is harmful, and the ethical requirement stands independently of any model's claims.

**The ethical question of the parts language.** A young person who has a religious or spiritual framework may hear the parts language in ways the practitioner does not intend, and the ethical requirement is to ask the young person how they understand the model and to use their framework where it is helpful. A young person from a family in which mental illness has been attributed to possession or to spiritual influence may find the parts model a relief or may find it an insult, and the practitioner cannot know which without asking.

**The ethical advantage of the model.** The model also has a substantial ethical advantage that should be recorded. For a young person who has been told that they are attention-seeking, manipulative, or defective, the model offers a description in which every part of their experience has a function, and in which the behaviour that has been most condemned is the behaviour that is most clearly a protector. That is not a small thing, and it is one of the strongest reasons for a student to know this model.

**The ethical requirement of the practitioner's self-monitoring.** The model's demand that the practitioner monitor their own state for blending is an ethical requirement, not a technical preference, and it is particularly important with young people, who are acutely sensitive to an adult's consistency and whose experience of the practitioner will determine whether the Exile work can be undertaken at all.

## 11. Key Terms

### Key Terms at a Glance

- **Internal family systems (IFS)** — A therapy developed by Richard Schwartz in which the mind is understood as a natural multiplicity of parts in relationship, with the symptom understood as the solution of a conflict among them.
- **Structural dissociation** — The theory, developed from Janet's work, in which overwhelming trauma produces a division of the personality into relatively autonomous action systems, one of which is fixated in the trauma.
- **Apparently normal part** — In structural dissociation theory, the part of a person that continues to function in the ordinary world and has no access to the traumatic material.
- **Emotional part** — In structural dissociation theory, the part fixated in the trauma and dominated by the traumatic affect.
- **Part** — A discrete aspect of a person's experience with its own perspective, emotional style, agenda, and history; a subpersonality with a legitimate adaptive function.
- **The Self** — The core level of experience in the IFS model, from which the practitioner works; described in the secular formulation through the 8 Cs.
- **The 8 Cs** — Curious, calm, compassionate, confident, connected, courageous, clear, committed; the descriptive qualities of the Self.
- **Blending** — The condition in which a part's perspective runs the show and the person is *in* a part rather than having one.
- **Unblending** — The development of the capacity to experience a part's feelings and thoughts while also experiencing oneself as the one who has them; the model's core technical idea.
- **Exile** — A part carrying the young person's wounds — the hurt, fear, shame, and injustice of earlier experiences — and often the youngest part.
- **Manager** — A proactive protector part whose strategies prevent the Exiles from being triggered; planning, controlling, achieving, suppressing, withdrawing.
- **Firefighter** — A reactive protector part whose strategies extinguish the Exile's activation; dissociation, bingeing, drinking, raging, self-harming.
- **Outsider** — A part that maintains the appearance of things and guards the system against the intensity of the work; includes scepticism and the part that wants to quit.
- **Symptom as solution** — The model's central clinical claim, that the symptom resolves the conflict among the parts and works, at a developmental cost.
- **Six-step process** — Mapping, connecting with Self, understanding protectors, healing the Exile, retrieving, and reworking.

**Key Terms at a Glance (continued)**

- **Retrieving** — The step in which the Exile gives back the burden it was given and is reoriented towards its original caretakers.
- **Reworking** — The final step in which the parts are reorganised so that the old strategies are no longer required.
- **Visual mapping** — The use of drawing to represent the system of parts; an important development in the child and adolescent applications.
- **IFS in schools** — A brief, educational adaptation of the model for use with young people in school settings.

## 12. Chapter Summary

Internal family systems therapy was developed by Richard Schwartz in the 1980s from two sources: his clinical observation of conflicted patients with eating disorders and substance abuse, and the theory of structural dissociation, which is itself derived from Pierre Janet's nineteenth-century account of how trauma divides the personality.

The model describes three levels of experience: the unblended Self, the parts, and the blending of a part's perspective into the person's everyday state. The five categories of part — Exiles, Managers, Firefighters, Self, and Outsiders — each have a distinct function, and the account of the symptom as the solution of a conflict among the parts is the model's most clinically powerful proposition.

The six-step process moves from mapping, through connecting with the Self and understanding the protectors, to healing the Exile, retrieving, and reworking. The concept of unblending gives the young person a choice they did not have in the moment, and the demand that the practitioner monitor their own state for blending is one of the theory's most valuable clinical disciplines.

The developmental fit with young people is good, and the model is uniquely compatible with the visual mapping that adolescents can work with, with the non-blaming account of the symptom, and with the developmental reality of a self that is under construction. The evidence base has grown substantially, with the strongest material in adult individual work and in the online delivery, and the family and school applications remain thinly evidenced.

The ethical requirements are strict and specific, particularly for the Exile work, where the structural dissociation literature's requirements for stabilisation are the ethical floor. The model's substantial ethical advantage is that for a young person who has been called defective or manipulative, it offers a description in which every part of their experience has a function.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the two origins of internal family systems therapy and explain the contribution each made to the model.
2. State the three levels of experience and explain the difference between blending and unblending.
3. Describe the function of each of the five categories of part, and give an adolescent example of each.
4. Explain the account of the symptom as a solution, and state the clinical corollary that follows from it.
5. What is the 8 Cs, and what is the secular formulation of the Self?
6. Describe the six-step process, and state the step at which the greatest risk is incurred with a young person.
7. Explain the distinction between a Manager and a Firefighter in terms of timing and direction, and illustrate it with a case.
8. What does the model contribute to the management of an adolescent in crisis?
9. What does the structural dissociation literature require before the Exile is approached, and why is that requirement ethical as well as clinical?
10. How might a young person with a religious framework understand the parts language, and what does the practitioner need to do about it?

## 14. Practice Activities

**Practice and Application**

### Activity 15.1 — Parts Mapping with a Peer (90 minutes, triads)

One student works on a real or imagined difficulty; one guides the mapping using the questions in this chapter; one observes. The observer's task is to note the moment at which the student uses the parts language fluently and the moment at which the language becomes a way of avoiding contact with the affect. The exercise builds the mapping skill and the self-monitoring skill simultaneously, which is appropriate, since the model requires them together.

### Activity 15.2 — The Manager/Firefighter Analysis (75 minutes, individual, written)

Take a young person you know and write a paragraph for the Manager and a paragraph for the Firefighter, using their own language where you can. Then write the paragraph for the Exile, and note how much you are willing to write about it. The disproportionate reluctance in the third paragraph is data about the young person and about you, and both are worth bringing to the seminar.

### Activity 15.3 — The Unblending Exercise (60 minutes, pairs)

One student reads aloud a short passage describing an internal conflict; the other repeatedly asks: "Who is saying that?" and "What is that part protecting?" The practitioner is required to keep asking until the student can name the parts, describe their functions, and step back from them. The point of the exercise is the moment of unblending, which is genuinely moving to experience and is worth understanding from the inside before attempting to facilitate it for someone else.

## 15. Annotated Reading Pathways

**For the model.** Read Schwartz (1995) and then the clinical guide, and read Brehm's workbook alongside them, because the workbook is the most usable account of the process with a young person.

**For the underlying trauma theory.** Read the structural dissociation material, and then read Janet's original work in a modern edition. The theory is more intelligible after Janet than before, and a student who has read Janet will find the contemporary somatic and trauma literature much easier.

**For the evidence.** Read the systematic reviews of the therapy in adults and the large online trials, and read the developmental and school-based studies with attention to their sample sizes and their developmental adaptations rather than to their conclusions.

**For the ethical questions.** Read the trauma literature on the dangers of premature trauma processing with young people, and the literature on the model's use in schools and in families. The school and family applications are where the ethical requirements are most easily breached, because the settings pressure for rapid symptom change.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Internal Family Systems and Structural Dissociation** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is schwarz's model of parts and the treatment of partitioned experience in traumatized youth. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is

helping rather than merely continuing. The organising focus for this part of the course is awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The mechanism to keep in view is how safety, attention, expression, and relational responsiveness allow experience to become available for reflection and choice. The characteristic professional risk is pressuring disclosure, treating emotional intensity as progress, or neglecting risk, family context, and practical action. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in individual sessions where trust, voice, and self-understanding are central, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Priya is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Priya's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Priya explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on awareness, emotional experiencing, relational presence, agency, and the young person's own meanings. The first intervention is deliberately modest. Priya chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Priya has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Priya can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Internal Family Systems and Structural Dissociation

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Internal Family Systems and Structural Dissociation** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Internal Family Systems and Structural Dissociation** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Internal Family Systems and Structural Dissociation**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Internal Family Systems and Structural Dissociation to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART III

# Cognitive Theories

The cognitive family of theories: the proposition that mental activity is mediated by the appraisal of events, the examination of belief systems, and the third-wave movement that loosened the grip of rational disputation.

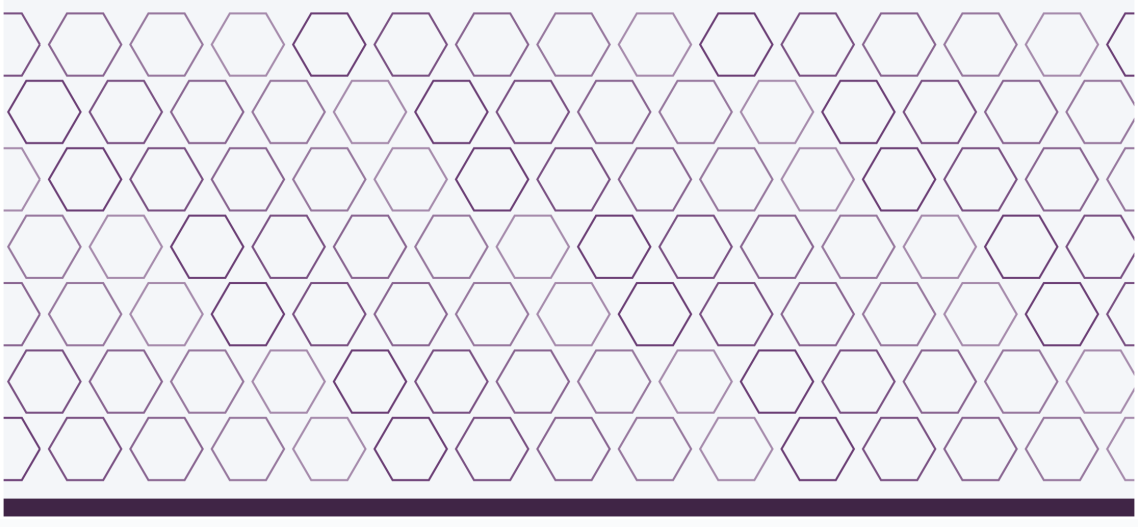
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CHAPTER 16

# Cognitive Therapy: The Cognitive Model

Aaron Beck's theory of depression and anxiety, the cognitive conceptualization triangle, and the disciplined practice of cognitive therapy.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the historical development of cognitive therapy, including the role of Aaron Beck's clinical research and the relationship of the theory to Kelly's personal construct theory.
- State the cognitive model in detail, including the levels of processing, automatic thoughts, intermediate and core beliefs, and the hot spots.
- Explain Beck's cognitive conceptualisation and describe the five components of a complete formulation.
- Describe the principal techniques of cognitive therapy and state the indications and cautions for each.
- Explain the structure of a cognitive therapy session and its rationale.
- Evaluate the evidence for cognitive therapy and for its efficacy in specific conditions, and state the limits of that evidence.
- Critically evaluate the cognitive model, including the treatment of the body, of emotion, of development, and of social and structural context.
- Apply the cognitive model to a case involving a child or adolescent.

## Chapter Overview

Cognitive therapy was developed by Aaron Beck in the 1960s, initially in the treatment of depressed outpatients at the Philadelphia Veterans Administration Hospital, and it was the first treatment in the history of the profession to be specified in a manual, to be tested in a controlled trial, and to be exported for use in other countries and other conditions. It is the approach against which most of the modern evidence-based practice movement was measured, and the "cognitive model" that it proposes is the most widely used explanatory framework in the profession.

The theory's central claim is that the way a person construes a situation determines the emotional and behavioural response to it, and that a great many of the difficulties that present for treatment are maintained by the way the person thinks about themselves, their situation, and their future. The therapeutic task is to identify the dysfunctional beliefs and to test them collaboratively, in a process that Beck called collaborative empiricism, in which the practitioner and the client are co-investigators in a set of propositions about which both can be, in principle, persuaded.

The chapter presents the model in detail, works through the cognitive conceptualisation and the technique repertoire, describes the session structure, and then addresses the evidence and the criticisms. The final section is a sustained critical appraisal, because the cognitive model's virtues are well documented and its weaknesses are, in the main, well documented too, and a graduate student should be able to state both.

## 1. Historical Context

### 1.1 Beck's research and the session model

Aaron Beck was trained in psychiatry and psychoanalytic psychotherapy, and he was in practice as a psychoanalytic therapist when he noticed something that disturbed him. His depressed patients, when asked to record their thoughts, produced material that was uniformly negative in tone — and, more strikingly, the

material was *specific and plausible*, not the vague, conventional self-criticism of the psychoanalytic literature. His patients said things like "I am no good," "I am a burden," "I will always fail," and they said them with conviction and with a logic that was internally coherent.

Beck's decision to shift his attention from the content of the transference to the content of the patient's thoughts in the session was the founding move of cognitive therapy, and the method he developed to elicit it — the **session model** — is the direct ancestor of the thought record and of every subsequent cognitive technique.

The method is worth describing precisely, because it is a technique for gaining access to the process rather than the content. Beck asked his patients, in the middle of a session, to report what was going through their minds — not what they were thinking about, but the thoughts themselves, in the form in which they occurred, including the images and the sensations. He then asked them to rate each thought on a scale of believability, and he asked them what emotional response each thought produced. He then asked them to consider what the evidence for and against each thought was.

Two findings followed. The first was that the negative thoughts were in fact **available to consciousness** — they were not buried, and they were not unconscious, and they did not require an hour of free association to surface. This was a significant challenge to the psychoanalytic account of the relationship between the presenting symptom and the underlying conflict, and it was a finding that the practice of cognitive therapy was designed to exploit. The second was that the beliefs underlying the negative thoughts were **widely held and held with conviction**, and that they were not in the first instance hypotheses to be tested but convictions to be examined. Beck called them "cognitive derivatives" and later "core beliefs."

## 1.2 Kelly and the personal constructs

The theoretical foundation of cognitive therapy is George Kelly's **personal construct theory**, published in 1955, and the debt is larger than is often acknowledged.

Kelly's claim was that each person constructs a personal theory of the world through a process of bipolar alternatives, and that a person is defined not by the content of these constructs but by the *way in which they construe events*. Two features of Kelly's theory are directly important to cognitive therapy.

The first is the **fixed-role therapy** approach: the client is invited to explore alternative construings of an event, and the exploration is treated as a laboratory in which the client's construing can be tested. This is the direct ancestor of the behavioural experiment.

The second is the emphasis on the **correlational context** of a construct: a personal construct is not a proposition in the abstract but a hypothesis about a correlation, and it is therefore testable. Kelly's formulation of a personal construct as a bipolar pair — "strong–weak," "responsible–irresponsible" — with a subordinate corollate, is the direct ancestor of the way cognitive therapists work with beliefs, and it is the reason that cognitive therapy uses the *form* of the belief rather than its content.

## 1.3 The naming of the model

The term "cognitive therapy" was coined by Beck and his colleagues, partly to distinguish the approach from "rational therapy" and partly to signal a break with the psychoanalytic tradition. The name was successful and the tradition grew quickly. By the 1980s, cognitive therapy was the most widely researched psychotherapy in the world, and by the 1990s the manualised treatments derived from it — including the treatments for anxiety and for depression — were the most widely used evidence-based treatments in the

world.

# 2. The Cognitive Model

## 2.1 Levels of processing

Beck's model describes three levels at which cognition may be analysed.

**The conscious level** consists of the thoughts that the person can report and that are available to reflection. It is the level at which most cognitive therapy is conducted, and it is the level at which the young person's own verbal capacity most directly constrains what is possible.

**The automatic level** consists of the rapid, habitual, and often unconscious appraisals that occur without the person's awareness and that generate the immediate emotional and behavioural response. Beck's "hot spots" are the specific automatic thoughts that are associated with the strongest emotional responses, and identifying them is the first technical task of a cognitive therapy session.

**The underlying level** consists of the more general and more deeply held beliefs, both intermediate and core, that give the automatic thoughts their structure and their power.

## 2.2 Automatic thoughts

An **automatic thought** is a self-directed internal statement that arises spontaneously in a situation, is experienced as true in the moment, and produces an emotional response. Automatic thoughts are not beliefs in the strict sense; they are situationally bound, momentary, and often adaptively functional. The young person who thinks "they are all watching me" on entering a room is having an automatic thought, not holding a belief, and the clinical point is that the difference matters: automatic thoughts are accessible, they can be examined, and they change.

The properties of automatic thoughts that matter clinically are that they are quick, that they feel true, that they are not usually noticed before the emotion, that they are often adaptive in the situations in which they arose, and that they are accessible to awareness once the practitioner points to them.

## 2.3 Beliefs

**Intermediate beliefs** are the conditional rules and assumptions that the person holds about situations, about themselves, and about other people, and that are derived from the core beliefs. They typically take the form of "if... then..." statements, and they are the level at which the young person's difficulties in specific situations are usually generated. The developmental literature is useful here: a young person's intermediate beliefs about school, about friends, and about their body are formed in the middle years and are highly specific to their contexts.

**Core beliefs** are the most general, most deeply held, and most difficult to change of the cognitive structures. Beck's classical categories are the three that have become the most familiar: **unlovable**, **worthless**, and **helpless**, with the addition of **inadequate** and **unfavourable-to-others** in later formulations. The categories are often presented as a list and the list is often used too readily. What matters is the specific content of the belief for a particular young person, and a core belief label is a shorthand for a richer clinical description that the practitioner must supply.

The properties of core beliefs are that they are global, that they are held as truths, that they are self-maintaining, that they are highly resistant to change, and that they are often held in a form the young person cannot articulate without assistance.

### 2.4 The cognitive triad

Beck's original formulation of the model was the **cognitive triad**: the belief that "I am worthless," the belief that "The world is unkind," and the belief that "My future is hopeless." The triad was the model as presented to the profession in the 1970s, and it has been substantially revised since, for three reasons: it is too simple for the complexity of the cases it is applied to; it does not describe the physiology of the response; and it does not accommodate the developmental and social context.

The revised formulation, in the contemporary model, is a hierarchical structure in which the core beliefs generate intermediate beliefs, which generate situation-specific automatic thoughts, which generate the emotional, behavioural, physiological, and cognitive responses. The three components of the older formulation are retained as "the heart, the core, and the key" of the model, but the model is no longer a triangle.

### 2.5 The hot spots

The **hot spot** is the specific situation in which a particular automatic thought produces the strongest emotional reaction, and identifying hot spots is the first technical task of the assessment and the organising structure of the therapy. The clinician's task is to find the situations in which the young person's automatic thoughts are most powerful, and to work with the thoughts in those situations, because it is in the hot spot that the cognitive system is most accessible to change.

## 3. The Cognitive Conceptualisation

Beck's **cognitive conceptualisation** is the systematic account of a case in the cognitive model, and it has five components. A student should be able to produce all five for a case.

**The core belief.** The most general belief the young person holds about themselves, their situation, or their future, and the evidence for it.

**The intermediate beliefs.** The rules and assumptions derived from the core belief, in the form of "if... then...", and the specific situations in which they operate.

**The automatic thoughts and hot spots.** The situation-specific thoughts, the emotional responses, and the point at which the emotional response is most intense.

**The behavioural and physiological responses.** What the young person does, and what happens in the body, in the hot spots.

**The coping behaviours.** The young person's strategies for managing the emotional response, and the consequences of these strategies for the maintenance of the difficulty.

The conceptualisation is not a static document. Beck's formulation is that the conceptualisation is a hypothesis that is tested in the session, and that the clinician should expect to revise it, and that a conceptualisation which does not change over the course of the therapy is a sign that the clinician has stopped attending.

### 3.1 The process of the conceptualisation

Three questions are asked in the session, and the answers constitute the conceptualisation.

**What are the words the young person uses to describe the situation?** The wording is diagnostic, because the young person's own vocabulary for their difficulty contains their cognitive structure. A young person who says "I can't" is describing a perceived incapacity; one who says "I won't" is describing a decision; one who says "I must" is describing a rule.

**What does the young person believe is going on?** The core belief is often most easily approached by asking what the young person would have to believe in order for their current situation to be what it is, and by asking what they would have to believe in order for things to be different.

**What are the consequences?** The formulation must specify the consequences of the belief in the domains of emotion, behaviour, and physiology, because the intervention is targeted at the consequences as much as at the belief.

## 4. The Technique Repertoire

### 4.1 The thought record

The thought record is the central technique, and it is worth describing with precision because the student will be expected to use it.

**The situation.** The specific situation, described in concrete, sensory terms and located in a particular place and time. The instruction to the young person is to describe the situation "as if you were watching it on film" or as if they were standing outside looking in, which draws attention to the observable and away from the interpretive.

**The emotions.** The emotions the young person felt, with their intensity rated, and the location of the emotion in the body.

**The automatic thoughts.** The thoughts that went through the young person's mind at the time, in the form in which they occurred, including images.

**The evidence for.** What supports the thoughts? This is a genuinely open question, and the practitioner should ask it as though the answer might support the young person's belief.

**The evidence against.** What does not support the thoughts? The difficulty here is that the young person frequently cannot generate the evidence against, or generates it and immediately neutralises it. The practical technique is to return to the actual situation and to ask what the young person would have expected to happen and what actually happened.

**Alternative thoughts.** What else could have been thought? The instruction is deliberately open: not "what is the rational thought?" but "what else could you have said to yourself?"

**The consequences of the alternative.** What would be different if the young person had held the alternative thought? This is a crucial step, because it converts a cognitive exercise into a motivational one: an alternative that makes no difference is not worth holding.

**The effect.** Rate the belief in the original thought now, and the belief in the alternative now.

The thought record is not a test of the young person's intelligence, and a practitioner who uses it as such will generate compliance rather than learning. The development of the technique with young people is the

subject of Chapter 17.

## 4.2 Behavioural experiments

A **behavioural experiment** is a planned test of a prediction derived from a belief, conducted in the world, with the result observed. The structure is: the prediction, the test, and the outcome, with the young person's actual experience of the outcome as the data.

The technical requirement is that the experiment be a genuine test — that it be capable of disconfirming the belief. An experiment designed to confirm the belief is not an experiment. This is the point at which the cognitive therapy practitioner most often goes wrong, and the discipline is to design the test with the young person and to ask explicitly, before the experiment, "what would we see if this belief turned out to be wrong?"

The developmental adaptation of the behavioural experiment is discussed in Chapter 17, and the two points to note here are that the experiment must be small and concrete for a young person, and that the outcome must be examined in detail rather than summarised, because a partial disconfirmation is more instructive than a complete one.

## 4.3 Agenda setting, mood monitoring, and the session structure

**Agenda setting.** The beginning of each cognitive therapy session is spent on the young person's agenda — what they want to work on. The rationale is that it establishes the young person's ownership of the work and it produces a co-constructed direction that the practitioner can then pursue. The practitioner's own agenda is deferred.

**Daily mood monitoring.** The young person records their mood across a number of activities each day, which provides the data for the between-sessions work and, importantly, gives the young person direct experience of the relationship between activity and mood, which is the empirical basis of behavioural activation.

**The session structure.** A cognitive therapy session has a recognisable structure: agenda setting, mood check-in, the work on the agreed agenda, homework assignment, and a summary. The structure is not bureaucratic; it is the mechanism by which the young person learns the shape of the work and by which the between-session transfer is secured.

## 4.4 Imagery rescripting and related techniques

**Imagery rescripting** is the modification of a mental image to change the emotional response to it, and it is used particularly with young people who have intrusive images of a frightening or humiliating event. **Decentring** is a technique in which the young person is taught to consider the full range of evidence and perspectives, so that the thought is held as a thought rather than as a truth. **The continuum of change** is a technique in which the young person is helped to see a belief as a continuum rather than as an absolute, and is used particularly with the rigid, dichotomous thinking that accompanies many of the presentations in youth services.

## 4.5 The exposure of core beliefs

The most advanced technique in cognitive therapy is the work with core beliefs, and it is done through a series of behavioural experiments and through the examination of the "person's data" — the evidence that the young person has about themselves. The procedure is to identify the core belief, to specify the predictions it generates, to test those predictions, and to construct a new, more qualified belief that accommodates the disconfirming evidence without discarding the adaptive core.

## 5. The Session Structure and the Working Alliance

The cognitive therapy session has a structure, and the structure is one of the reasons the treatment is deliverable in a brief or time-limited form. The session is agenda-led, which means that the young person determines the content, and the clinician's role is to bring the cognitive model to bear on whatever the young person has brought.

The **alliance** in cognitive therapy has been studied in detail, and the findings are consistent with the general alliance literature: the alliance predicts outcome in cognitive therapy as it does in other therapies, and the alliance mediates the delivery of the specific techniques. Two features of cognitive therapy that are relevant to the alliance are worth noting.

The first is the **collaborative stance**, which is the defining interpersonal feature of the approach. The clinician does not tell the young person what to think; they examine the evidence together, and they are as prepared to be persuaded as the young person is. Beck's formulation of this is that the clinician and the young person are co-investigators, and that the therapeutic relationship should feel like a scientific inquiry.

The second is the **guided discovery**, which is the method by which the collaborative stance is enacted. The clinician asks questions, and the questions are designed to help the young person find the answer rather than to deliver it. The question "what evidence do you have for that?" is the prototype. The risk of guided discovery is that it becomes a way of avoiding a direct statement that the young person needs, and a clinician who has mastered the technique can be almost impossible to argue with, which is a problem with a young person who has a legitimate grievance about the clinician.

## 6. Cognitive Therapy in Other Conditions

The cognitive model was developed for depression, and it was applied to anxiety within a few years, with the result being the manualised cognitive-behavioural treatments for generalised anxiety, panic disorder, social phobia, and specific phobia, and later for PTSD, for eating disorders, for psychosis, for depression in young people, and for a range of developmental and health conditions.

Two developments are worth noting for a student in this course. The first is the development of the **transdiagnostic** model, in which the same cognitive mechanisms — overestimation of threat, overestimation of uncontrollability, and overestimation of the unfavourability of the self, and the corresponding attentional biases — are proposed to operate across disorders, which has led to a unified treatment for a broad range of conditions. The second is the development of the **third wave** approaches discussed in Chapters 20 to 22, which reformulate the cognitive model to emphasise the function of thoughts rather than their content.

## Further Reading for This Chapter

- Beck, A. T. (1976). *Cognitive therapy and the emotional disorders*. International Universities Press. — The primary text; the model and the cognitive conceptualisation.
- Beck, A. T. (1988). *Cognitive therapy of depression*. Guilford. — The fullest treatment of the technique with a particular condition.
- Beck, J. S. (2011). *Cognitive behavior therapy: Basics and beyond* (2nd ed.). Guilford. — The standard clinical text and the one a student should work from.
- Persons, J. B., & Tompkins, M. A. (Eds.). (2014). *Psychotherapy case formulation*. — Includes an account of the cognitive conceptualisation in the context of the wider formulation literature.
- Padesky, C. A. (1994). *From thought to change*. Guilford. — The clearest account of the collaborative empiricism and the guided discovery.
- Driessen, M. (2014). *Doing what matters in clinical practice: A guide to CBT*. — A practitioner-oriented account that is unusually honest about the difficulties.

## 7. Cognitive Therapy with Children and Adolescents

Chapter 17 develops the developmental modifications in full. This section provides the outline that the full chapter follows, so that the reader can see the shape of the developmental argument before encountering it in detail.

**The four modifications.** Cognitive therapy with a young person requires modification in four areas: the level of processing at which the work is done, the language and the medium of the thought record, the degree of developmental approximation involved in the behavioural experiments, and the involvement of the parent or other adults.

**The developmental prohibition and its limit.** A young person below approximately twelve cannot reliably produce abstract propositional thoughts, cannot reliably introspect into automatic thoughts, and cannot reliably evaluate alternative beliefs against evidence. The modification is to work with concrete, situational, imaginal, and behavioural material, and the limit is that the work is then closer to behavioural therapy than to cognitive therapy in its adult form.

**The problem of the doubting mind.** A young person who has learned to doubt everything, and who has been told by adults that their thoughts are "not true," will not accept the collaborative empiricism on which the approach depends. The modification is to establish the collaborative stance through action rather than through argument — that is, through the joint conducting of behavioural experiments whose results the young person can observe.

**The parent.** The developmental literature supports parental involvement, and the cognitive therapist must be able to work with a parent who is a well-meaning but anxious participant in the young person's difficulties. Chapter 17 sets out the specific modifications.

## 8. Modern Developments

### 8.1 The transdiagnostic model

The transdiagnostic cognitive model, developed principally by Susan Muse and colleagues, is the most significant theoretical development in the cognitive tradition since Beck's original formulation, and it has a direct relevance to a youth service because it addresses the co-occurrence of problems rather than treating them as separate.

Its central claim is that a limited number of cognitive processes are shared across the emotional disorders: the overestimation of threat, the overestimation of uncontrollability, and the overestimation of the unfavourableness of the self. The accompanying treatment, the unified protocol, is a transdiagnostic cognitive therapy delivered in a flexible number of sessions.

The relevance to this course is that it matches the clinical reality of a youth service, where a young person frequently presents with several difficulties at once — anxiety, low mood, a conduct problem, and substance use — and where the traditional approach of treating each diagnosis with its own protocol is impractical. The transdiagnostic approach makes it possible to work on the shared processes and to address the co-occurrence directly.

### 8.2 The third wave and the relational turn

The third wave approaches discussed in Chapters 20 to 22 reformulated the cognitive model in two fundamental ways: they shifted the target from the content of thoughts to the function of thoughts, and they made the therapeutic relationship the primary intervention. The **relational cognitive therapy** tradition, associated with Jeremy Holmes and with the work that follows the maintenance model of the alliance, has made a complementary move: it locates the young person's core beliefs in the relationship with the practitioner, and it treats the alliance rupture as the primary occasion for core belief work.

This last development is particularly relevant with young people, because it does not require the young person to have the verbal capacity to produce propositional thoughts, and because it uses the relationship itself as the data.

### 8.3 The computerised and digital delivery

The digital delivery of cognitive therapy is now a substantial evidence base in its own right, and the findings are highly relevant to youth services: the digital treatments produce modest but real effects, they are acceptable to young people, and their acceptability is partly a function of the fact that they remove the need for the young person to disclose to an adult. The digital cognitive therapy for anxiety and for depression in young people is among the most evaluated of all psychological interventions, and the numbers of young people reached are an order of magnitude greater than those reached by any face-to-face service. Chapter 55 discusses the ethical and practice implications.

## 9. Evidence and Research Support

### 9.1 The strength of the evidence

The cognitive-behavioural therapies have the largest and most methodologically careful evidence base of any treatment in the profession, and a student should be able to specify it.

**For depression in young people.** The evidence for the manualised cognitive therapy for depression in adolescents and young adults is among the strongest in the child and adolescent literature, with multiple

large randomised controlled trials, systematic reviews, and meta-analyses, and with a well-established developmental adaptation. The effects are moderate to large, and the treatment is the recommended first-line treatment for depression in young people in most international guidelines.

**For anxiety.** The evidence for the cognitive-behavioural treatments of anxiety in young people is likewise strong, and the adaptations for children — with the exposure components, the modelling, the parent involvement, and the concrete behavioural work — have been tested in large trials.

**For specific phobias and for panic.** The evidence is strong in adults and good in young people, and the exposure-based components have an unusually clear mechanism.

**For the transdiagnostic protocol.** The evidence is smaller but growing and includes randomised controlled trials in young people.

**For the alliance and the mechanism.** The process literature is substantial: the alliance predicts outcome in cognitive therapy, the alliance mediates the delivery of the technique, and the in-session rating of the young person's perceived helpfulness and the therapist's rating of the young person's receptiveness are among the more reliable predictors of session outcome.

## 9.2 The limits of the evidence

**The population problem.** The adolescent trials have included more diverse samples than the earlier adult literature, but the over-representation of white, affluent, female, and non-medically-ill participants persists, and the evidence for young people with neurodevelopmental conditions, with intellectual disability, with chronic physical health conditions, with severe and multiple mental health difficulties, and with justice involvement remains much thinner.

**The developer and allegiance problem.** A substantial proportion of the key trials in the adolescent depression literature were conducted by investigators with a strong allegiance to the treatment, and a re-analysis of the trials of a widely used manualised treatment for adolescent depression that excluded the developer-investigator trials found a substantially smaller effect. The finding has been widely discussed and is important for the student to know: the effect size in the independent trials is smaller than the effect size in the developer-led trials, though it remains clinically meaningful.

**The brevity of the follow-up.** A number of the key adolescent depression trials have relatively short follow-up periods, and the longer-term questions — about maintenance, about the developmental trajectory of the young person after the treatment ends, and about the effect on the developmental tasks the young person is working on — are largely unanswered.

**The generalisability question.** The evidence is strongest for the treatment of a defined disorder with a defined protocol, delivered by trained practitioners to a selected population, and it is considerably weaker for the "practical, integrated treatment" that most services actually deliver.

## 9.3 What the evidence does not support

That cognitive therapy is the treatment of choice for every young person with a cognitive presentation. That the cognitive model is a complete account of how young people become depressed or anxious. That the mechanisms proposed are the only mechanisms operating. Or that the equivalence of outcomes across therapies implies that the cognitive model is as good a framework as any other — the equivalence finding is a statement about the absence of a general hierarchy, not about the interchangeability of models in a given

case.

## 10. Strengths and Limitations of the Cognitive Model

**Strengths.** It provides a clear and testable account of how difficulties develop and are maintained, and the account is one that can be checked rather than merely believed. It generates a coherent technique repertoire in which every intervention is connected to a mechanism, and this is the practical source of the treatment's efficiency and its manualisability. The collaborative empiricism is a genuinely respectful relational stance that suits a young person's developmental interest in being treated as a thinker rather than as a patient. The evidence base is the largest in the field, the treatment is deliverable in brief forms, and the digital versions reach young people who will not come to a clinic. The technique repertoire transfers into many other frameworks.

**Limitations.** The model is thin on the body, on emotion as an independent phenomenon, and on the developmental and relational embedding of the young person's thinking. Its account of the person is individualistic, and its account of learning is almost entirely propositional, which excludes the young people who do not think in propositions. The early versions of the model were developed and tested on adults and the developmental adaptations were added later, and the evidence for the adaptations is thinner than the evidence for the original. The model has a well-documented tendency to produce compliance rather than learning, particularly with young people who have learned that adults want a particular answer. And the cognitive model has been, historically, the least attentive of the major therapies to the social and structural determinants of a young person's difficulties.

## 11. Cultural and Ethical Considerations

### 11.1 The cultural location of the cognitive model

The cognitive model assumes that the young person is capable of a reflective, propositional, individual, and largely verbal process, and it assumes that the beliefs that maintain the difficulty are individual, internal, and appropriately open to examination. Several elements of this assumption are culturally marked.

**The individual and the autonomous thinker.** The model's figure is a person who generates beliefs, examines them, and revises them. In cultural contexts in which the self is relational, in which a young person's account of what is "true" is understood to be a matter of shared agreement rather than of private appraisal, and in which revision of one's beliefs is a public and socially consequential act, the collaborative empiricism may be experienced as intrusive rather than as respectful.

**The treated vs. the treating dynamic.** The collaborative empiricism presupposes a symmetry between the practitioner and the young person in the examination of evidence. Where the practitioner holds social, institutional, and possibly legal authority over the young person's life, this symmetry is nominal. The ethical requirement is to make the asymmetry explicit, and the practitioner's own account of a problem should be offered for examination alongside the young person's.

**The demand for verbal and introspective competence.** A young person who is not verbally fluent, who is neurodivergent, or who comes from a home in which the explicit articulation of belief is not modelled will struggle with the model's central procedure. The ethical requirement is that the practitioner's method adapt to the young person and not the other way about, which in practice means abandoning the thought record

format for something more concrete while retaining the underlying discipline of collaborative examination.

### 11.2 The ethical requirement of not blaming

The most important ethical question raised by the cognitive model is the one it has been most often accused of: that the cognitive account of a young person's difficulty can become a claim that the young person is responsible for their own distress.

The distinction the student must hold is between an account of how a belief was formed and a judgement about the person who holds it. The cognitive model is an account of formation, and the clinical work is explicitly to trace the belief to its origins in a context in which the young person had little choice — a family in which the belief was taught, a school in which it was confirmed, a world in which it was accurate. The ethical requirement is that the tracing is done and is done genuinely, and that the practitioner does not allow a young person to end a course of treatment believing that they have simply failed to think correctly.

A second ethical requirement follows: the cognitive model is not an appropriate frame for difficulties that are produced by injustice, and a practitioner who applies it to a young person who has been racially abused at school, or who is in an unsafe home, or who is living in poverty, will produce a young person who blames themselves for a situation in which they had no part. The practitioner must hold the two accounts together, and the test of whether they have done so is whether the young person leaves the treatment with an accurate account of what happened to them.

### 11.3 The ethical requirements of the collaborative stance

**Informed consent to the examination of beliefs.** A young person should know what cognitive therapy involves, including that their beliefs will be examined and that the practitioner will offer alternative accounts. This is a real requirement and it is not usually met, because the method is presented as a conversation rather than as an intervention.

**The requirement of a right answer.** A young person should be able to decline to change a belief, and the practitioner must be able to work with a young person who holds a belief that is, on the evidence, accurate. A young person who believes that the school is unsafe because it is unsafe should not be helped to think otherwise.

**The requirement of competence.** The procedures of cognitive therapy are not neutral instruments. The guided discovery, the thought record, and the behavioural experiment can all be experienced as intrusive, and the practitioner must be competent in their delivery, must attend to the young person's response, and must be willing to abandon a procedure that is not working.

## 12. Case Illustration: The Thought Record and the Young Woman Who Could Not Say No

**The referral.** Aiko, seventeen, is referred by a college tutor for "panic and depression." Aiko has missed twenty-one of her forty-two scheduled classes. She has a long history of "anxiety" and has been prescribed medication twice, which she stopped on both occasions without telling her GP. She describes herself as "a worrier." She is bright, has a place at university conditional on grades she is not achieving, and her parents expect her to attend a medical school in her home country.

**The cognitive conceptualisation.** The conceptualisation is built over the first three sessions, and the practitioner works from the session model — asking Aiko to report what went through her mind in specific moments rather than asking her to describe her feelings in general.

The **hot spot** is the tutor's email. The **automatic thought** is: "Everyone will think I am stupid and a let-down." The **emotion** is a sharp, hot feeling in the chest and throat, and the **behaviour** is to not open the email, to avoid the tutor, and to miss the class.

The **intermediate beliefs** are: "If I make a mistake, people will judge me as a failure" and "If people judge me, I will not be able to survive it." The **core belief** is: "I am fundamentally inadequate." The **coping behaviours** are avoidance, over-preparation in a way that never quite arrives at adequacy, and, in the sessions themselves, a compliance with the practitioner that the practitioner notices and names.

**The compliance.** In the fourth session the practitioner says: "I want to check something with you. When I asked you yesterday what you thought about missing the class, you said 'I don't know, it just happened'. Three days ago you told me a very precise story about what you were thinking in the corridor. Which of those is the real one?" Aiko is quiet. Then: "The second one. I just didn't want to be a difficult client."

**The therapeutic work.** The work has three strands.

The first is the **behavioural experiment**, and it is designed collaboratively and rigorously. Aiko believes that if she asks a question in class, people will judge her as stupid. The experiment: she asks one question in a small seminar group, in a way agreed in advance, and the practitioner specifies the predictions and the observations in advance. Aiko's prediction: "They'll think I'm stupid and they'll remember it." The agreed observation: "Note what actually happens — who says what, and what they do next." The outcome: two people answer her question normally and one other student asks one back. The practitioner then insists on the detailed examination: "Your belief predicted that they would think you stupid and remember it. What actually happened?" Aiko says: "They didn't." The practitioner does not press the point further, which is a discipline the technique requires.

The second is the **thought record**, and it is adapted. Aiko is seventeen and can do a written thought record, so she does the standard version, and the practitioner reviews it weekly. Two features are added. The first is that the record is completed in the session, in part, so that the practitioner can observe the process; the second is the "what does the world need to look like for this belief to be true" column, which is the technique that connects the cognitive work to the systemic level.

The third strand is the **core belief work**, and it is here that the model's limits appear. Aiko's core belief of inadequacy is thoroughly supported by her developmental history: she was the family disappointment, the one who did not speak the family's language well, the one whose educational achievements were interpreted as compensating for something. A cognitive therapist who works only with the present will produce a young woman who knows better but does not feel better. The practitioner therefore works explicitly on the origin of the belief, in a manner closer to the psychodynamic than to the classic cognitive tradition, and in the sessions the practitioner begins to look at what happens between them, because the young woman's relationship to the practitioner is rapidly reproducing the relationship with the tutor.

**The developmental and cultural questions.** Three.

The developmental question: Aiko is at the stage at which the identity task is in progress, and the possibility of a life that is not the family's plan is a developmental task and not a symptom. The practitioner asks what Aiko wants, and discovers that Aiko does not know, and that this not-knowing is a developmental achievement rather than a deficit. The cognitive work on the belief "I am fundamentally inadequate" is

therefore also the work of making room for the developmental task of choosing.

The cultural question: Aiko's family has a plan, the plan involves a return to the home country, and the plan is not a choice that Aiko has made. The practitioner asks the family question directly: "What does your daughter need to be?" and the answer, when the mother is asked, is that she needs to be a doctor who can support the family. The practitioner does not proceed further with the family work at that point, and records the decision and the reason, which is that the differentiation of Aiko from the family's project has to wait until Aiko has enough of her own sense of what she wants to be able to survive the conversation.

The ethical question: Aiko's medication has been stopped twice without the GP knowing. This is a safeguarding and a coordination issue that the practitioner raises directly with Aiko, and it is handled as a collaborative problem rather than as a disclosure.

**The outcome.** Over four months, the attendance rises to seventy per cent, the panic episodes reduce, and Aiko continues to be treated. The core belief is not resolved and is unlikely to be; Aiko describes it as "still being there, but quieter." The practitioner records this as a partial and honest outcome and continues the treatment at a reduced frequency. The last session in this phase is spent on Aiko's own formulation of her position, which she writes out and which the practitioner keeps with Aiko's permission. It is a good formulation. It is not a cognitive one, and the practitioner notes that in the record.

## 13. Key Terms

### Key Terms at a Glance

- **Cognitive model** — Beck's account in which the way a person construes a situation determines the emotional and behavioural response to it.
- **Session model** — Beck's method of eliciting and evaluating thoughts in the middle of a session, the ancestor of the thought record.
- **Automatic thought** — A self-directed internal statement arising spontaneously in a situation, experienced as true, and producing an emotional response.
- **Hot spot** — The specific situation in which a particular automatic thought produces the strongest emotional response; the organising structure of the assessment.
- **Intermediate belief** — A conditional rule or assumption derived from a core belief, operating in specific situations.
- **Core belief** — A global, deeply held, and highly resistant belief about oneself, one's situation, or one's future.
- **The cognitive triad** — Beck's original formulation of unhelpful beliefs about the self, the world, and the future; the model of which the triad was the heart.
- **Cognitive conceptualisation** — The five-component account of a case: core belief, intermediate beliefs, automatic thoughts and hot spots, behavioural and physiological responses, and coping behaviours.
- **Collaborative empiricism** — Beck's term for the joint investigation of propositions, in which the practitioner and the client are both prepared to be persuaded by the evidence.
- **Guided discovery** — The method by which the collaborative stance is enacted through questions that help the young person find the answer rather than delivering it.
- **Thought record** — The structured record of a situation, the emotions, the automatic thoughts, the evidence for and against, the alternative thoughts, and the consequences.
- **Behavioural experiment** — A planned test of a prediction derived from a belief, conducted in the world, whose outcome is the data.
- **Decentring** — The technique of holding a thought as a thought by considering the full range of evidence and perspectives.
- **Continuum of change** — The technique of helping a young person see a belief as a continuum rather than as an absolute, used with dichotomous thinking.
- **Imagery rescripting** — The modification of a mental image in order to change the emotional response to it.

**Key Terms at a Glance (continued)**

- **Transdiagnostic model** — The account in which a limited number of cognitive processes operate across the emotional disorders.
- **Unified protocol** — A transdiagnostic cognitive therapy designed to be delivered flexibly in a limited number of sessions.
- **The maintenance model** — The account in which the young person's core beliefs are maintained in and through the therapeutic relationship, and in which the alliance rupture is the primary occasion for core belief work.
- **Convergence of therapy and medicine** — The practice of treating the body's physiological state as a component of the cognitive work, developed in behavioural medicine and in health psychology.
- **Equivocality** — The quality of a young person's thinking that is a normal and developmentally appropriate feature of the adolescent's mind and is frequently pathologised by adults.

## 14. Chapter Summary

Cognitive therapy was developed by Aaron Beck in the 1960s in the treatment of depression, and it is the first treatment in the profession's history to have been manualised, tested in a controlled trial, and exported. Its theoretical foundation is George Kelly's personal construct theory, and its central method is the session model.

The model describes cognition at three levels: the conscious, the automatic, and the underlying. The automatic thoughts are situationally bound and adaptive; the intermediate beliefs are the conditional rules that generate difficulty in specific situations; the core beliefs are the global and deeply held convictions that give the others their structure. Beck's original cognitive triad has been replaced by a hierarchical model, and the technical organisation of the work is around the hot spot.

The cognitive conceptualisation has five components — core belief, intermediate beliefs, automatic thoughts and hot spots, behavioural and physiological responses, and coping behaviours — and it is a hypothesis to be tested in session rather than a document to be filed. The technique repertoire follows from the model: the thought record, the behavioural experiment, the agenda-setting session structure, and the more advanced work with core beliefs through behavioural experiments and the examination of the young person's evidence about themselves.

The session is agenda-led and the relationship is collaborative, and the empirical literature on the alliance in cognitive therapy confirms what the theory predicts: the alliance mediates the delivery of the technique. The modern developments — the transdiagnostic model, the relational turn, and the digital delivery — have extended the model's reach considerably, and the last of these is reaching young people who would never attend a clinic.

The evidence base is the largest in the field, and the limits of that evidence are substantial: the population problem persists, the developer-allegiance problem has reduced the estimated effect size in independent trials, and the follow-up periods are often short. The model's limitations are equally real: it is thin on the body, on emotion, on development, and on the social determinants, and it has a documented tendency to

produce compliance rather than learning. The ethical requirements that follow are the examination of the young person's belief with genuine openness, the requirement to trace the belief to its origins, and the requirement not to use the model to explain away injustice. The case of Aiko shows the model at its most effective and the point at which it requires supplementation.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe Beck's session model and state the two findings that followed from it.
2. Explain the contribution of Kelly's personal construct theory to cognitive therapy, using the bipolar construct and the fixed-role approach.
3. State the three levels of the cognitive model and give an example of each with a young person.
4. Distinguish an automatic thought from a belief, and explain why the distinction is clinically useful.
5. Distinguish intermediate beliefs from core beliefs, and state the properties of a core belief.
6. Explain why the cognitive triad was replaced by the hierarchical model.
7. State the five components of the cognitive conceptualisation.
8. What is collaborative empiricism, and what is the risk of the guided discovery that enacts it?
9. Describe the structure of a cognitive therapy session and state the rationale for each element.
10. What is the developer-allegiance problem, and what effect did it have on the estimated effect size of a widely used treatment for adolescent depression?

## 16. Practice Activities

**Practice and Application**

### Activity 16.1 — The Hot Spot Map (90 minutes, individual, then pairs)

Choose a recent difficult situation from your own life. Reconstruct the moment in the concrete, sensory terms of the session model — what you saw, what you heard, what you were thinking, in the form in which the thoughts occurred. Identify the automatic thought, the emotion, and its bodily location. Then write the intermediate belief and the core belief beneath it. Bring the map to the seminar. The exercise is uncomfortable and it is the foundation of every later clinical skill in this course.

### Activity 16.2 — The Experimental Design Clinic (2 hours, small groups)

Each group takes a belief that is amenable to a behavioural experiment and designs the experiment in detail: the prediction stated in a form that could be disconfirmed, the specific test, the agreed observations, and the analysis of what would count as disconfirmation. The group must also specify what the young person would do differently if the belief were true. The groups then swap designs and critique them, looking specifically for experiments that are incapable of disconfirming the belief. The critique is the exercise.

### Activity 16.3 — The Compliance Check (60 minutes, pairs, recorded)

One student role-plays a practitioner using guided discovery; the other plays a sixteen-year-old who is compliant and who wants to please. The practitioner is instructed to notice the compliance aloud. Then swap, and the "practitioner" in the second round is instructed to notice the difference in the young person's response. The purpose is to make the compliance of a young person visible, which is a skill that is difficult and that most adults find almost impossible to do without considerable practice.

## 17. Annotated Reading Pathways

**For the model and the technique.** Read Beck (1976) for the model, Beck (1988) for the technique with depression, and Beck (2011) for the current clinical practice. Read Padesky (1994) alongside them for the collaborative empiricism, which is the conceptual heart of the approach and is usually under-taught.

**For the developmental work.** Read Chapter 17 of this book and then the developmental depression and anxiety literatures, including the treatment adaptation literature. Note what the adaptations preserve and what they sacrifice, and note the cost of the adaptations in terms of the evidence base.

**For the critique.** Read the health psychology literature on the cognitive model in physical health conditions, the transdiagnostic literature, and the critical literature on the cultural assumptions of the cognitive model. The model is more robust than its critics allow and more limited than its proponents claim, and the student should be able to hold both.

**For the mechanism.** Read the process research on the alliance in cognitive therapy and on the mediators of outcome. This is the evidence that connects the theory to the outcome and that reconciles the cognitive and common factors positions.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Cognitive Therapy: The Cognitive Model** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is aaron Beck's theory of depression and anxiety, the cognitive conceptualization triangle, and the disciplined practice of cognitive therapy. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what

to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The mechanism to keep in view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tunde is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tunde's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tunde explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Tunde chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tunde has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tunde can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Cognitive Therapy: The Cognitive Model

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Cognitive Therapy: The Cognitive Model** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Cognitive Therapy: The Cognitive Model** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

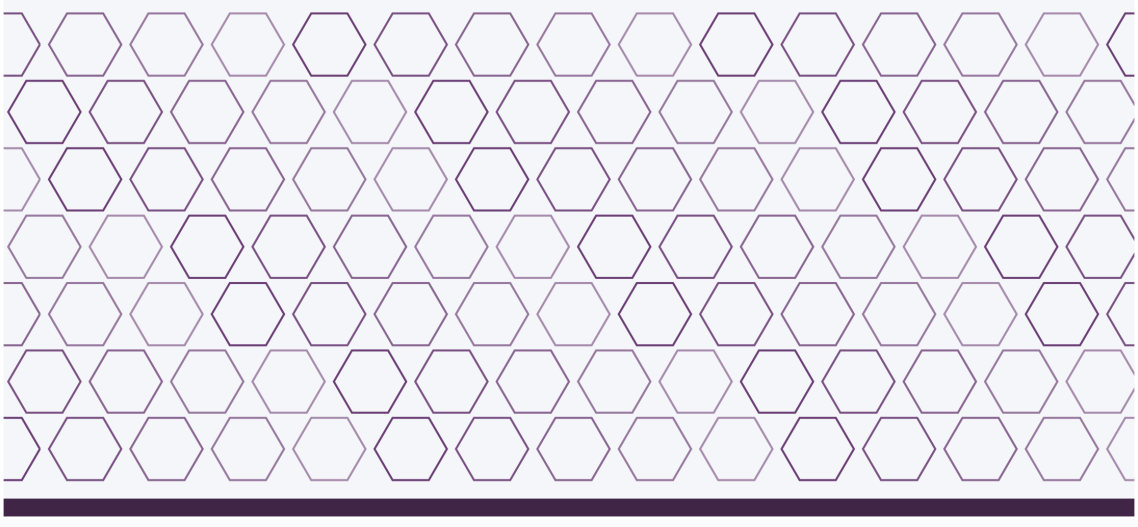
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Cognitive Therapy: The Cognitive Model**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Cognitive Therapy: The Cognitive Model to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 17

# Cognitive Therapy with Children and Adolescents

Developmental adaptations, the role of the parent, and the modification of thought records for younger clients.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain why the cognitive model as developed with adults cannot simply be applied to a young person, and identify the specific developmental obstacles.
- Describe the developmental modifications required to the thought record and state the rationale for each.
- Design developmentally appropriate behavioural experiments for a child, an early adolescent, and a late adolescent.
- Work effectively with parents of young people receiving cognitive therapy, and describe the specific pitfalls.
- Describe the adaptations of cognitive therapy for the treatment of depression, anxiety, and OCD in young people.
- Apply the cognitive model to a developmental formulation, and identify where a developmental or systemic account is required.
- Evaluate the evidence for developmentally adapted cognitive therapy with young people.
- Identify the specific cultural and ethical issues raised by the developmental adaptation of the cognitive model.

## Chapter Overview

The cognitive therapy developed by Beck was for adult outpatients, and the adaptations required for children and adolescents are not cosmetic. A fourteen-year-old cannot reliably produce abstract propositional thoughts; a nine-year-old cannot reliably introspect into an automatic thought; a seventeen-year-old who has learned to distrust adults' suggestions will comply with the collaborative empiricism while privately rejecting it. A student who applies the adult thought record to a young person will produce a session in which the young person writes down a false statement about a moment that never happened, and the practitioner will believe it is a gain.

This chapter works through the developmental problem in detail and then develops the modifications: of the thought record, of the behavioural experiment, of the parent's role, of the work with the young person's developmental task, and of the relationship between the cognitive work and the body. It also examines the three conditions in which cognitive-behavioural treatments have been most extensively adapted and evaluated — depression, anxiety, and obsessive-compulsive disorder — and it is candid about the limits of the adaptation in each.

The central argument of the chapter is that the developmental modification of a therapy is not a dilution of it. The cognitive model contains a discipline — the collaborative examination of propositions — that is more important than the specific format of the thought record, and the adaptation consists in finding formats that preserve the discipline while fitting the young person's capacity. A practitioner who can do that is doing cognitive therapy with a young person; a practitioner who cannot is doing something else, and should know which.

## 1. The Developmental Problem

## 1.1 The obstacles

Four specific developmental obstacles stand between the adult cognitive model and a young person.

**The obstacle of propositional thought.** Cognitive therapy's central procedure requires the young person to (a) identify a situation, (b) identify the thoughts that occurred in it, (c) state them in a form that can be evaluated, (d) generate alternatives, and (e) assess the alternatives against evidence. The first two steps are within the capacity of most young people from about eight years old. The third requires propositional thought, which is a late-developing capacity and one that is materially affected by arousal, fatigue, and mood. The fourth requires the ability to generate a divergent response to a problem, which is one of the more demanding of the executive functions and one of the most sensitive to the young person's belief that the problem is hopeless — which is to say, the very belief the treatment is addressing. The fifth requires the ability to hold an evaluative stance toward one's own thinking, which is precisely what is impaired in the conditions the treatment targets.

This is the fundamental paradox of cognitive therapy with young people, and the adaptation must address it directly: the very capacities the technique requires are the capacities the condition has impaired.

**The obstacle of introspection.** Automatic thoughts are by definition not noticed before the emotion. A young person at the concrete operational stage who is asked to report what they were thinking in a situation three days ago will construct an answer rather than recall one, and the construction will be a plausible account assembled from the available social narratives about what people think in such situations. This is not deception; it is the developmental limits of retrospective introspection.

**The obstacle of the doubting mind.** A young person who has been told, by parents, by teachers, and by other adults, that their thoughts are "not true" or "not real" or "just a phase" has learned a lesson about the reliability of their own cognition that will interfere directly with the collaborative empiricism. The lesson is not irrational; it has usually been well-founded.

**The obstacle of the power differential.** The collaborative empiricism presupposes a symmetry of attention between the practitioner and the young person. A young person who is in the care of adults, or who is under school discipline, or who has been referred by a parent who is present, cannot assume that the examination of propositions is a joint activity. They will, often without awareness, provide the answers that will keep them safe.

## 1.2 The developmental solution

Each of these obstacles has a corresponding adaptation, and the adaptations are the substance of this chapter.

**Developing the capacity rather than requiring it.** The developmental solution to the obstacle of propositional thought is to treat the capacity as something to be taught within the treatment, which is what the developmental adaptations do: they teach the young person to notice, to label, to evaluate, and to generate, in a sequence, over the course of the sessions.

**Using behavioural and imaginal material rather than verbal material.** The developmental solution to the obstacle of introspection is to work with material that does not require retrospective verbal recall. A young person can be asked what they were feeling and what was happening in their body in a specific recent situation, and can often produce concrete detail about it, even when they cannot produce a thought record.

**Establishing the collaborative stance through action.** The developmental solution to the doubting mind is to demonstrate the joint examination rather than to assert it: to conduct behavioural experiments in which the young person predicts, the test is run, and the outcome is observed by the young person.

**Working with the parent as a co-investigator.** The developmental solution to the power differential is to make the parent a participant in the collaborative examination rather than an observer of it, in the specific ways set out in Section 4.

# 2. The Developmental Model of Cognition

## 2.1 Vygotsky and the zone of proximal development

The most useful theoretical tool for adapting cognitive therapy to a young person is the zone of proximal development. A cognitive task that is too difficult produces helplessness and withdrawal; a task that is too easy produces boredom. The task must be pitched so that the young person can complete it with the support of the practitioner, and the support must then be withdrawn progressively.

The developmental adaptations of cognitive therapy are, in effect, a systematic application of this principle: the young person is not asked to produce a full thought record alone but is helped to produce the components in sequence, with the practitioner modelling the process at first and then fading the support.

## 2.2 Piaget and the operational stages

The concrete operational stage is the stage in which the young person can think logically about concrete material, can take another's perspective, and can conserve. The formal operational stage brings the capacity for abstract and hypothetical reasoning. The developmental adaptations of cognitive therapy map onto these stages with reasonable accuracy, and the mapping is a useful guide for the practitioner.

For the **concrete operational child (approximately 7–11)**, the cognitive work is conducted with concrete situations, with drawings and images, with behavioural experiments that are simple and physical, and with the immediate and the observable. The core belief work is done not as a proposition but as a *picture* and a *feeling*, and the language of the work is "the rule that goes with the picture" rather than "the belief."

For the **early adolescent (approximately 11–13)**, the cognitive work can begin to use propositions, but the propositions must be about concrete and recent situations, and the young person's own words rather than the practitioner's must do the work. The behavioural experiments can be more elaborate and can involve social situations.

For the **middle and late adolescent (approximately 13–18)**, the formal operations are available and can be used, but they are fragile in the presence of strong affect, and the practitioner should plan to do the cognitive work when the young person is not in the middle of the difficulty. This is the point at which the "pre-agreed coping plan for the moment of difficulty" ceases to be a supplement and becomes a precondition.

## 2.3 The development of self-representation

The developmental literature on the self provides the account of why some of the presentations in youth services are so difficult to treat with a cognitive model, and the account is worth understanding.

**The concrete operational child** has a self that is largely constituted by observable actions and by the judgments of others. The child is "good" if others say so, and the cognitive work is about the difference

between what the child has done and what others have said, and the child can often make this distinction more readily than an adult expects.

**The early adolescent** has a self that is becoming explicitly a *possible* self, and the adolescent's central developmental work is the reconciliation of the actual self with the possible self. The discrepancy between the two is the source of a great deal of adolescent anxiety and depression, and it is not a "cognitive distortion" in the sense of an inaccurate appraisal; it is an accurate description of a developmental gap. A cognitive therapist who treats it as a distortion is treating a developmental fact as an error.

**The mid-to-late adolescent** is developing a more abstract, more integrated, and more self-authored self-concept, and the developmental work is the integration of the contradictory elements. The developmental literature on this period describes it as a process that is considerably more turbulent than the popular account of the "storm and stress" suggests, and it is a period in which a great many young people consult mental health services.

# 3. The Developmental Thought Record

## 3.1 The five-column version

The standard thought record has seven columns, and the developmental version reduces and simplifies them. The typical developmental thought record for an adolescent has four or five columns.

**What happened?** A concrete, sensory description of the situation, in the young person's own words, and often with a drawing or a diagram.

**What went through your mind?** The automatic thought, in the young person's words, and with the option of a drawing of the thought. The instruction is deliberately simple and the practitioner accepts the answer without requiring elaboration at first.

**How did you feel?** The emotion, named from a visual or written list, with a rating and, if the young person is willing, the location in the body. The visual list of emotions is a substantial practical improvement on the adult version and is easily produced.

**What is the evidence for and against?** For younger children, this is often a concrete question about what happened and what the young person saw, and the "against" column is frequently completed by the practitioner as a collaborative activity, which is itself a demonstration of the collaborative stance.

**What could you have said instead?** For younger children, this is often expressed as an alternative *action* rather than an alternative thought: "What else could you have done?" The action formulation is developmentally easier and is a bridge to the behavioural experiment.

## 3.2 The "said-to" adaptation

A well-established developmental adaptation of the thought record for children and younger adolescents is the **"said-to" version**, in which the young person is asked to complete a set of sentence stems about what they *said* rather than what they thought: "My friend said that..." "My teacher said that..." "I said to myself that..." The "I said to myself" column then becomes the working automatic-thought column.

The adaptation is effective because it moves the young person from a metacognitive task they cannot perform to a recall task they can, and because the speech attribution gives the thought a location in the world that makes it available for examination. A young person who has written down what their teacher said can

then examine the belief, because it is now on the page.

### 3.3 The "worst, best, most likely" adaptation

For the behavioural experiment with a young person, the "worst, best, and most likely" prediction scale is a useful developmental adaptation of the standard technique. It provides a structure, it is concrete, and it produces the crucial insight — that the young person's own estimate of the likely outcome is much less catastrophic than their estimate of the worst — without the practitioner having to argue for it.

### 3.4 The cautions

**Do not let the thought record become a test.** The most common error with young people is the drift into a performance of the technique, in which the young person produces increasingly sophisticated entries and learns nothing. The test for this is whether the young person's affect has changed in the session. If it has not, the practitioner has been collecting data rather than working.

**Do not require written work from a child who cannot do it.** A nine-year-old's thought record can be a drawing, a comic strip, a voice recording played back by the practitioner, or a conversation the practitioner records afterwards. The discipline is in the examination of the evidence, not in the format.

**Do not let the record become a compliance exercise.** A young person who has learned that adults want a particular answer will produce it. The practitioner who does not notice is doing therapy on a collaboration rather than on a young person.

## 4. Working with Parents

### 4.1 The case for parental involvement

The developmental and outcome literatures support parental involvement in the treatment of young people's anxiety, and there is a substantial body of evidence that parent involvement in the treatment of adolescent depression improves outcomes and reduces relapse. The reasons are developmental: the parent is a more powerful agent of change in the young person's environment than the therapist is; the parent's own beliefs and responses maintain the young person's difficulties; and the work is a developmental task in which the parent has a developmental role.

### 4.2 The specific cognitive work with parents

The cognitive work with parents is not the same as the cognitive work with the young person, and a student should be able to distinguish four elements.

**The parent's own beliefs about the young person's symptoms.** Parents of young people with anxiety and depression frequently hold beliefs about the cause of the difficulty — that it is "attention-seeking," that it is "a phase," that "it will pass," that it is caused by screens or by a friendship group — and these beliefs are often themselves the subject of a cognitive analysis. The work is to examine the parent's beliefs with the same collaborative empiricism, and to do it without implying that the parent's beliefs are foolish.

**The parent's responses.** The parent's responses to the young person's anxiety or distress are the proximate maintenance factors in many cases: reassurance, avoidance of the feared situation, allowing the young person to avoid school, and the accommodation of the symptom. The cognitive therapist does not tell the

parent to stop accommodating; the therapist helps the parent examine what they believe the accommodation is for, which is usually love and usually a reasonable response to the young person's distress.

**The behavioural experiments for the parent.** The parent's own predictions — about what will happen if the young person goes to school, about what will happen if they stop reassuring — can be tested in the same way as the young person's, and this is often a very effective intervention with a parent who is sceptical about the whole enterprise.

**The parent's own difficulties.** Parents of young people with serious difficulties are frequently depressed, anxious, or in unresolved conflict with each other, and the therapeutic work with the parent may need to be the more substantial of the two.

### 4.3 The specific pitfalls

**The triangulation.** The most serious pitfall in parent-inclusive work is the covert alliance between the practitioner and the parent against the young person. A practitioner who works with the mother and then interprets the young person's difficulty to the mother in the young person's presence has enacted the very dynamic that the young person came to escape. The requirement is that the practitioner's work with the parent and the young person be kept structurally separate, and that the young person be told what will be shared.

**The parent who is the problem.** The developmental literature on the family is clear that in a substantial minority of cases the parent's own difficulties, the family's emotional patterns, or the parent's unresolved history are the maintaining factors. A cognitive therapist who works only with the young person's appraisals is doing something in these cases, but is not doing the whole thing.

**The parent who will not engage.** A parent who does not attend, or who attends and is not able to change anything, is a common situation. The requirement is that the practitioner be explicit with the young person about what is and is not possible, and that the young person not be blamed for the parent's absence.

**The young person who does not want the parent involved.** A young person who is seventeen and who has told the practitioner that the session is confidential will not want the parent in the room, and the developmental and ethical requirement is that the young person's preference is respected, subject to the safeguarding and the legal requirements, and that the practitioner explain to the parent what the work involves without disclosing the content.

## 5. The Three Best-Evidenced Adaptations

### 5.1 Depression in young people

The developmentally adapted cognitive therapy for depression in young people is among the most extensively evaluated psychological treatments in the world, with multiple large randomised controlled trials in the United States, the United Kingdom, Australia, and New Zealand.

The modifications are: the inclusion of a substantial behavioural activation component, including the scheduling of activities to reverse the withdrawal and the anhedonia; the involvement of parents, in a graduated fashion; the use of a simplified and more concrete cognitive model, with the "evidence for and against" work often done collaboratively and with the young person; the attention to the social and academic consequences of the depression, which are frequently more modifiable than the cognitive content; and the

explicit framing of the treatment as a skill to be learned and used in the future, which is developmentally consonant with the adolescent's interest in competence.

The clinical caution is that the behavioural activation component in adolescents must be paced carefully: the adolescent's activity schedule is a developmental task, and the research on the efficacy of activity scheduling with young people indicates that pushing too hard produces a further drop in mood and a further withdrawal.

### 5.2 Anxiety in young people

The adaptations here are the most thoroughly developed, because the anxiety disorders are the conditions in which the cognitive-behavioural approach has been most consistently successful with young people, and because the disorders have the clearest and most reliably replicated treatment components.

The modifications are: the use of a graded exposure hierarchy, with the young person and the practitioner constructing it collaboratively, and with the exposure being concrete and behaviourally defined; the use of modelling, with the practitioner or a peer demonstrating the feared activity; the reduction of parental accommodation, with a graded and collaborative approach; the use of the young person's own hierarchy rather than a standard one; and the use of reward systems for completed exposures, which is developmentally appropriate and which Chapter 28 develops in the school context.

The psychological model in these treatments has been developed specifically to be developmental: the developmental model of anxiety proposed by the developmental psychopathology researchers, in which the child's fear is a rational response to a real lack of control in a specific domain, and in which the treatment is the development of the skills of control and the reduction of the parental response to the fear. Chapter 46 develops this.

### 5.3 Obsessive-compulsive disorder in young people

The adaptation of exposure and response prevention to children and adolescents is a further major development, and it is a demanding one, because the treatment involves a young person deliberately approaching what terrifies them.

The modifications are: the caregiver's role in the exposure hierarchy and in the support for the response prevention; the developmental preparation and the psychoeducation for the young person; the specific attention to family accommodation; the attention to the young person's developmental understanding of the obsessions; and the modification of the response prevention for the young person's developmental stage, including the involvement of a parent in refraining from accommodating.

## 6. Emotion, the Body, and the Developmental Model

Two limitations of the adult cognitive model become particularly consequential in work with young people, and a student should be able to name them.

**The thinness of the emotional account.** The adult cognitive model treats emotion as a response that follows from appraisal, and its techniques are directed at the appraisal. The developmental and clinical literatures on young people suggest that for many young people the most accessible and most useful level of intervention is the emotional one, and that a young person who is asked to appraise before they can feel will not manage the appraisal. The modifications that respond to this are: the beginning of the emotional work

before the cognitive work in the session; the explicit teaching of the emotion vocabulary with a visual list; the attention to the bodily location of the feeling as a route into the cognitive work; and the use of the young person's own metaphors.

**The absence of the body.** The cognitive model has, historically, had little to say about physiological arousal, and a great many of the young people who present with panic or with somatic symptoms have a body that is a substantial part of the problem. The modifications are: the explicit inclusion of interoceptive awareness work; the practice of grounding; the attention to sleep; the physiological components of the treatment for panic; and the integration of the somatic work of Part VI.

The modern convergence between the cognitive and somatic traditions is a direct response to this weakness, and a student who is familiar with the somatic material in Part VI will be able to improve their cognitive work with young people considerably.

## Further Reading for this Chapter

- Beck, J. S. (2011). *Cognitive behavior therapy: Basics and beyond* (2nd ed.). Guilford. — Its chapters on children and adolescents, and on the adaptations, are the clinical core.
- Reinecke, L., & Roberts, S. (2004). *Cognitive therapy for children and adolescents*. — The best single clinical account of the developmental adaptations.
- Kirk, S., & Kutchins, H. (2013). *Cognitive-behavioral therapy for adolescents*. Guilford. — Detailed and practical.
- Weisz, J. R., & Lau, J. (2013). *The effects of psychotherapy for depression in children and adolescents*. — The developmental meta-analytic literature.
- Nichter, M. J., & Telch, M. J. (2017). *The prevention and treatment of anxiety disorders in children and adolescents*. — The anxiety adaptation literature.
- Scoville, K. T., et al. (2017). *A meta-analysis of outcome measures and a review of treatment effects in pediatric OCD*. — The OCD adaptation literature.

# 7. The Developmental Formulation

## 7.1 From the cognitive conceptualisation to the developmental formulation

The cognitive conceptualisation of Chapter 16 is necessary but insufficient with a young person, because it locates the difficulty inside the young person's thinking and locates the developmental context nowhere. A developmental formulation adds four things.

**The developmental position.** The formulation states where the young person is in relation to the tasks of their stage, and what the difficulty consists of in developmental terms. A formulation that says "Nadia's core belief is that she is unlovable" is incomplete; the developmental formulation adds that Nadia is at the stage at which the identity task is in progress, that the core belief has the function of protecting against the developmental risk of committing to an identity that will be disapproved of by her family, and that the belief is thus not merely inaccurate but developmentally costly in a way that a cognitive intervention alone will not

reach.

**The developmental trajectory.** The formulation states the expected course with and without intervention, and it states whether the developmental task is progressing.

**The level of the intervention.** The formulation states at what level the cognitive work should be conducted — the concrete, the imaginal, or the propositional — and this is a technical decision with clinical consequences.

**The developmental and structural context.** The formulation states what the young person's development is being obstructed by, beyond the young person's own thinking: the school's response, the family's pattern, the material conditions, the discrimination.

### 7.2 A worked formulation

Take Kofi, fourteen, who is referred for "school refusal and low mood." The cognitive conceptualisation establishes: the hot spot is the maths lesson; the automatic thought is "everyone will see that I can't do it"; the emotion is a hot, tight feeling in the chest; the behaviour is to leave the room; the intermediate belief is "If I make a mistake in front of people, I will be humiliated"; the core belief is "I am stupid"; the coping behaviours are avoidance and reassurance-seeking from his mother.

The developmental formulation adds the following.

*The developmental position.* Kofi is in the school-age-to-adolescent transition, and the industry-versus-inferiority task is the relevant one; the avoidance is serving to prevent the confirmation of the "inferiority" position, and the confirmation is being actively engineered by the school. The avoidance is not a failure of courage; it is a rational response to an environment in which the risk of humiliation is high and repeated.

*The trajectory.* The trajectory is towards increasing withdrawal, narrowing of the social world, and a rising risk of the young person being classified and streamed, with the developmental consequence that the school leaving outcomes and the young person's sense of competence both deteriorate. The developmental literature on school refusal and on chronic absenteeism is consistent about this trajectory and about the narrowing of the young person's developmental world.

*The level of the intervention.* The cognitive work is at the concrete and imaginal level, with a graded behavioural experiment at the centre, and the young person's mother is involved with the first two steps of the accommodation reduction.

*The context.* The school has a streaming policy that places Kofi in a lower set, the mathematics teacher has a reputation for public correction, and Kofi's mother's anxiety about his school attendance has led to a series of health appointments for "tiredness" and "stomach aches" that have not been examined for their function. The formulation therefore specifies that the work is not only with Kofi's appraisal but with the school, with the mother, and with the physical symptoms.

The result is a formulation with a different plan from the cognitive one alone: the graded exposure of the mathematics classroom, the work with the teacher about the public correction, the graded reduction of the mother's checking and reassurance, and the medical review of the stomach aches, alongside the cognitive work on the "I am stupid" belief.

## 8. Evidence and Research Support

**The evidence is strongest for the developmentally adapted treatments of depression and anxiety, and it is stronger than for any other psychological treatment with young people.**

**Depression.** The developmentally adapted cognitive therapy for depression in young people has been evaluated in multiple large randomised controlled trials across several countries, with a meta-analytic pooled effect in the moderate range, and with a re-analysis excluding developer-investigator trials producing a smaller but still clinically meaningful effect. The evidence for parental involvement, for the combination of behavioural activation with the cognitive work, and for the treatment delivered in school settings is good. The evidence for the long-term maintenance of the gains is thinner, and the follow-up data in several trials show a partial erosion of the treatment effect over the following year, which has led to the development of maintenance and booster strategies and to the current preference for the treatment of depression as a longer-term and developmentally embedded piece of work.

**Anxiety.** The developmental adaptations of the cognitive-behavioural treatments of anxiety are among the most evaluated interventions in the child mental health literature, and the effect sizes are moderate to large. The parent involvement component has been demonstrated to be a mediator of outcome in several trials, and the exposure and modelling components carry the effect.

**Obsessive-compulsive disorder.** The adapted exposure and response prevention has a strong evidence base in young people, with large effects, and with a body of research on the family accommodation component and on the developmental predictors of outcome.

**The limitations.** The population problem persists in all three literatures. The samples are disproportionately white, female, affluent, and without neurodevelopmental conditions or chronic physical illness, and the evidence for young people with these characteristics is much thinner, and in the case of neurodevelopmental conditions the treatments frequently need substantial modification, with the modifications often under-researched. The generalisability of the trials to ordinary services is a question that the pragmatic and practice-based literature raises, and the developmental adaptations have not been subjected to the same scrutiny that the adult treatments have.

**The evidence for the developmental adaptations of the thought record as such is much thinner than the evidence for the treatments that contain them.** The field has generally evaluated packages rather than components, and a student should be aware that the specific claim "this format of the thought record is effective with this age group" is much less well supported than "this package is effective."

## 9. Strengths and Limitations of Developmental Cognitive Therapy

**Strengths.** The cognitive model provides a testable account that a young person and a family can engage with, and the collaborative empiricism is a relational stance that is compatible with the developmental requirement that a young person be treated as a thinker. The evidence base is the strongest in the field, the treatments are manualised and deliverable in schools and in brief forms, and the digital versions reach young people who will not attend a clinic. The approach is teachable, and a graduate student can learn the core procedures competently within a year.

**Limitations.** The model is thin on emotion, on the body, on development, and on the social and structural context, and each of these thinnesses is consequential with young people. The developmental adaptations are considerable, and the packages that are evaluated are complex enough that the practitioner must be

trained in them rather than simply applying the adult model. The approach can produce compliance rather than learning, and the compliance is more difficult to detect with a young person than with an adult. And the historical tendency of the field to treat the young person's thinking as the site of the problem, and therefore to locate the solution in the young person's head, has had measurable consequences in the services in which it has dominated.

# 10. Cultural and Ethical Considerations

## 10.1 Culture in the developmental adaptation

The developmental modifications of the cognitive model are themselves culturally marked in ways that are easy to miss. The "safer" version of the young person is assumed to be a young person who can tolerate the examination of their thinking, and the techniques that are used to promote this — the thought record, the behavioural experiment, the exposure, the mood chart — are all techniques that presuppose a young person who is willing to be observed and to have their predictions tested.

Several of the young people for whom the evidence is weakest are those for whom these presuppositions are least secure: a young person who has been racially profiled by a school, a young person who is undocumented, a young person who has been told by an authority that their account is not believed, a young person with a communication difference who has been told that they "express themselves badly." For these young people, the collaborative empiricism must be established with unusual care, and the practitioner's first task may be to establish that the examination of propositions will not be used against them.

The emotional vocabulary list developed for the developmental adaptation is also culturally specific. The list is derived from English-language emotion research conducted largely with Western participants, and the categories in it — the distinctions among "ashamed," "embarrassed," and "guilty," for example — do not map cleanly onto the emotional categories of every language. A practitioner working with a young person who is more comfortable in another language should develop the vocabulary with the young person and their family rather than impose the list.

## 10.2 The ethical question of the behavioural experiment with a young person

The behavioural experiment is the technique with the greatest potential for ethical difficulty in the cognitive tradition, because it deliberately arranges for the young person to encounter the feared situation and to be exposed to the risk that the belief is confirmed.

The ethical requirements are: that the experiment be genuinely designed to be capable of disconfirming the belief; that the young person understand what will happen and why, in a form they can grasp; that the young person have a genuine choice about whether to proceed; that the practitioner be confident that the support is adequate if the experiment is difficult; that the consequences of a disconfirmation be considered in advance; and that the young person not be exposed to a risk that the practitioner has not assessed.

The most common breach is the fourth. An experiment that is too ambitious, conducted to demonstrate the treatment's effectiveness rather than to test the belief, is a common error in services with outcome targets, and it is the developmental equivalent of the premature confrontation discussed in Chapter 11.

## 10.3 The ethical question of the parent

The ethical requirements in work with parents are specific. The young person must know what will be shared with the parent, and the practitioner must be able to work with a parent without disclosing the young person's disclosures. The requirement of confidentiality with minors is a legal requirement that varies by jurisdiction, and the practitioner must know the law in the jurisdiction in which they practise. The requirement against the covert alliance between the practitioner and the parent is an ethical requirement and not merely a technique. And the requirement that the young person's own account of their difficulties be treated as primary, even when the parent and the practitioner agree about it, is the requirement that most often fails in practice.

### 10.4 The ethical advantage

The ethical advantage of the cognitive approach with young people is worth recording. It offers a young person a way of understanding and of influencing their own experience, which is a considerable gift, and it does so in a framework in which the young person is the investigator and the adult is the collaborator. For a young person who has been told that they are the problem, the model says: you are the one who has been studying this problem, you have collected a great deal of data, and some of your conclusions need revisiting. That is a very different conversation from the one the young person has usually had.

## 11. Case Illustration: The Thought Record That Was Not a Thought Record

**The referral.** Priya, eleven, is referred by her school for "anxiety and refusing to do PE." Priya's mother is a single parent of three, working full time, and Priya is the eldest. The school reports that Priya has become "withdrawn in PE", that she has been "crying" twice, and that she has refused on four occasions. There is a note in the file from the school counsellor: "P talks about feeling 'different' and says her skin feels wrong. Mum says she's always been a worrier."

**The first session.** The practitioner does not begin with the thought record. She begins with a body outline that she has drawn on a large sheet of paper, and she asks Priya to colour in the places where she feels things. Priya colours her chest and her stomach. The practitioner asks, "And when you are in the changing room and the other girls are talking, what happens?" Priya says: "My tummy goes like this." She makes a gesture. The practitioner asks her to draw that shape on the body outline. Priya does, and the shape is a tight spiral.

**The work.** Over the next three sessions, Priya and the practitioner work with a series of small, concrete, sensory-level "thought records" that are adapted for a child who is eleven and who has never been asked to introspect about her thinking. The record has four questions, written in Priya's own words on a card that she keeps in her pocket:

1. *What was happening?* (Priya draws it.)
2. *What did your body do?* (Priya circles on the body outline.)
3. *What did you think?* (Priya writes one sentence, or draws a word balloon.)
4. *What did you do next?*

The fourth question is the one that produces the most change, and it produces it because the behavioural link becomes visible. Priya's pattern: something in the changing room is said — often something about her body,

and specifically about her skin, which is darker than some of the other girls' — and then Priya's stomach tightens, and then she goes to the toilet and stays there until the lesson is nearly over, and the consequence is that she is relieved and that the other girls leave her alone.

**The formulation.** The cognitive conceptualisation is: the hot spot is the changing room; the automatic thought is "they're talking about me"; the intermediate belief is "if they see my skin they will talk about it and I will be humiliated"; the core belief is "I am different and that is bad"; the coping behaviour is avoidance, and it works.

The developmental formulation adds: Priya is at the stage at which the peer group is becoming the primary developmental context and the developmental task of the school-age years — the acquisition of competence and the establishment of a peer position — is being obstructed by the avoidance, which is removing her from the very context in which the task is performed; the avoidance is also removing her from the physical education, which is where she is physically competent, and the developmental cost of that is the loss of a domain of mastery at exactly the age at which a domain of mastery is being established.

And the formulation specifies the level of the intervention: the cognitive work is at the concrete, imaginal, and behavioural level, and the work is with the school, because the reference to Priya's skin is not a distortion to be corrected but a description of what is happening in the changing room.

**The work with the school.** The practitioner contacts Priya's PE teacher and asks a functional question: "What happens in the changing room?" The teacher is surprised and thinks about it, and then says: "Oh — we talk about our skin. The other girls, they talk about their skin, and I don't think they notice, but Priya's in the middle of it." The teacher, who has never thought about it, changes what happens: the girls are asked to change in a particular way, or the changing is staggered, or the class is asked to stop talking about their bodies. No one needed to be accused of anything.

**The outcome.** Priya returns to PE. Her "thought records" become more verbal over the weeks, and she begins to write a fifth question on her card: "What could I try?" The core belief does not shift entirely, and the practitioner does not press. The significant change is behavioural and contextual, and the practitioner records the case accordingly.

**The teaching point.** The cognitive work was not the difficult part and was not the most important part. The "thought record" was a four-question card with a body outline and a spiral, and it was not a thought record at all in the adult sense. The most important interventions were the body outline, which gave a non-verbal young person access to her own experience, and the question to the teacher, which changed the environment in which the difficulty was being maintained. A practitioner who had applied the adult thought record to Priya would have produced a sophisticated-looking record in which the hot spot was the changing room, the automatic thought was mis-stated, the core belief was described in adult language, and nothing at all would have changed.

## 12. Key Terms

### Key Terms at a Glance

- **Developmental adaptation** — The modification of a treatment's procedures to fit a young person's cognitive, emotional, social, and autonomy capacities; a condition of validity rather than a dilution.
- **Propositional thought** — The capacity to think in abstract propositions; late-developing, arousal-sensitive, and required in the standard form of the cognitive techniques.
- **Retrospective introspection** — The ability to report one's own thinking in a past situation; a developmentally limited capacity on which the adult thought record depends.
- **The doubting mind** — The learned distrust of one's own thinking that results from having been told that one's thoughts are not true, and that interferes directly with the collaborative empiricism.
- **The "said-to" thought record** — A developmental adaptation in which the young person reports what was said rather than what was thought, shifting the task from metacognition to recall.
- **The four-question record** — A simplified developmental thought record in a child's own words, typically covering the situation, the body, the thought, and the next action.
- **The body outline** — A pictorial instrument for locating feelings and sensations in the body; a developmental alternative to the verbal emotion rating.
- **Worst, best, most likely** — A developmental adaptation of the behavioural experiment's prediction rating, which permits the young person to discover that their own estimate of the likely outcome is less catastrophic than their estimate of the worst.
- **Developmental model of anxiety** — The account in which a child's fear is a rational response to a real lack of control in a specific domain, and in which the treatment develops control and reduces the caregiving response to the fear.
- **Parent accommodation** — The parent's reinforcement of the young person's avoidance and reassurance-seeking; a primary maintenance factor and a primary target in the anxiety adaptations.
- **Behavioural activation with young people** — The scheduling of activity to reverse the withdrawal and anhedonia of depression; requires careful pacing to avoid a further drop in mood.
- **Streaming** — The school practice of grouping students by attainment; a systemic factor that must be addressed in the formulation of a young person's academic difficulties.
- **The optimisation of the problem** — A specific cognitive therapy technique in which the problem is imagined to be as bad as possible, used with catastrophising.
- **Socratic questioning** — The technique of examining a belief by asking questions rather than by making statements; a cognitive technique that is a poor fit with a pre-operational child.

**Key Terms at a Glance (continued)**

- **The mood chart** — A daily record of mood and activity across a range of activities, used both to generate behavioural activation and to provide between-session data.
- **Public correction** — A classroom practice in which a teacher corrects a student publicly; a specific and modifiable maintaining factor in the academic anxiety of a young person.
- **Cognitively mediated maintenance** — The account in which the young person's own cognitive processes maintain the disorder, as distinct from the accounts in which the maintaining factors are primarily external.

## 13. Chapter Summary

The adult cognitive model cannot be applied to a young person without substantive modification, and the reason is a paradox that the adaptations must address: the very capacities the technique requires — propositional thought, retrospective introspection, evaluative self-stance, divergent thinking — are among the capacities the condition has impaired, and several of them are late-developing and arousal-sensitive.

The developmental solutions are systematic: develop the capacity rather than require it, use behavioural and imaginal rather than verbal material, establish the collaborative stance through demonstrated action rather than through assertion, and involve the parent as a co-investigator rather than as an observer.

The developmental adaptations of the thought record are concrete and simple: the "said-to" version, the four-question record in the young person's own words, the body outline, and the worst–best–most-likely prediction scale. The work with parents has four distinct elements — the parent's beliefs about the symptom, the parent's responses, the parent's own predictions, and the parent's own difficulties — and four characteristic pitfalls, of which the covert alliance with the parent is the most serious.

The three best-evidenced adaptations are for depression, anxiety, and obsessive-compulsive disorder, and the evidence in each is strong, though the population limitations are consistent and the follow-up data in the depression literature show a partial erosion of the effect. The developmental formulation adds the developmental position, the trajectory, the level at which the intervention should be conducted, and the structural context, and the case of Priya shows what the adaptation looks like in practice: a body outline, a four-question card, and a telephone call to a teacher who had never thought about what happened in the changing room.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. State the four developmental obstacles to applying the adult cognitive model to a young person, and the developmental solution for each.
2. Explain the paradox at the centre of cognitive therapy with young people, and state how the adaptations address it.
3. Apply the zone of proximal development to the design of a thought record for a nine-year-old.
4. Describe the "said-to" adaptation and state the reason it works.
5. List the four elements of the cognitive work with parents, and distinguish them.
6. Describe the four pitfalls of parent-inclusive work, and state which is the most damaging and why.
7. State the specific modifications in the treatment of adolescent depression, and explain why behavioural activation must be paced.
8. Explain the developmental model of anxiety and its implications for the treatment of a young person who is afraid of the school changing room.
9. What does the developmental formulation add to the cognitive conceptualisation, and why is each of the four additions clinically necessary?
10. In the Priya case, why would the adult thought record have produced nothing, and what did the body outline do that it could not?

## 15. Practice Activities

**Practice and Application**

### Activity 17.1 — The Four-Question Record (90 minutes, pairs)

Design a four-question thought record for a nine-year-old, a twelve-year-old, and a sixteen-year-old, in each case using language the young person would actually use. Then take the youngest version and use it on yourself, in relation to a real moment from the past week. The exercise develops the design skill and the bodily self-awareness, and the two are not separable.

### Activity 17.2 — The Parental Beliefs Audit (75 minutes, individual, written)

Take a young person you know well, or a memory of one. Write down the beliefs the parent holds about the young person's difficulty — at least five, in the parent's probable words. Then, for each belief, write: what does this belief protect the parent from? The exercise is the cognitive work with parents in miniature, and it is usually the first thing a trainee understands about parental involvement.

### Activity 17.3 — The Compliance Watch (90 minutes, triads)

One student plays a practitioner working with a young person who is being compliant; one plays the young person; one observes for any moment at which the practitioner could have noticed the compliance. The observer's task is to note the specific verbal or behavioural cues that signal it, and to note what the practitioner did. Then reverse the roles. The skill being built is the detection of performance, and it is one of the hardest in the course.

## 16. Annotated Reading Pathways

**For the clinical adaptations.** Read Reinecke and Roberts (2004) and Kirk and Kutchins (2013), and use Beck (2011) alongside them for the adult procedures. The gap between the two is instructive in itself, and a student who has read both will not accidentally import the adult format.

**For the developmental rationale.** Read the developmental meta-analyses for depression, anxiety, and OCD, and read the treatment adaptation literature rather than the treatment manuals. The adaptations are where the developmental understanding is visible, and the manuals tend to present them as settled technique rather than as theoretically informed modifications.

**For the parent work.** Read the parent involvement literature in the anxiety and depression trials, and read the family systems literature in Part V, which approaches the parent's own difficulty from a different direction.

**For the critique.** Read the literature on the treatment of young people with neurodevelopmental conditions and with chronic physical illness, and see how the model's assumptions fail and how the adaptations have been made. This is where the next generation of developmental cognitive work is being done.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Cognitive Therapy with Children and Adolescents** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is developmental adaptations, the role of the parent, and the modification of thought records for younger clients. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The mechanism to keep in view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically

correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Sofia is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Sofia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Sofia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal,

belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Sofia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Sofia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Sofia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.

- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Cognitive Therapy with Children and Adolescents

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Cognitive Therapy with Children and Adolescents** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Cognitive Therapy with Children and Adolescents** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

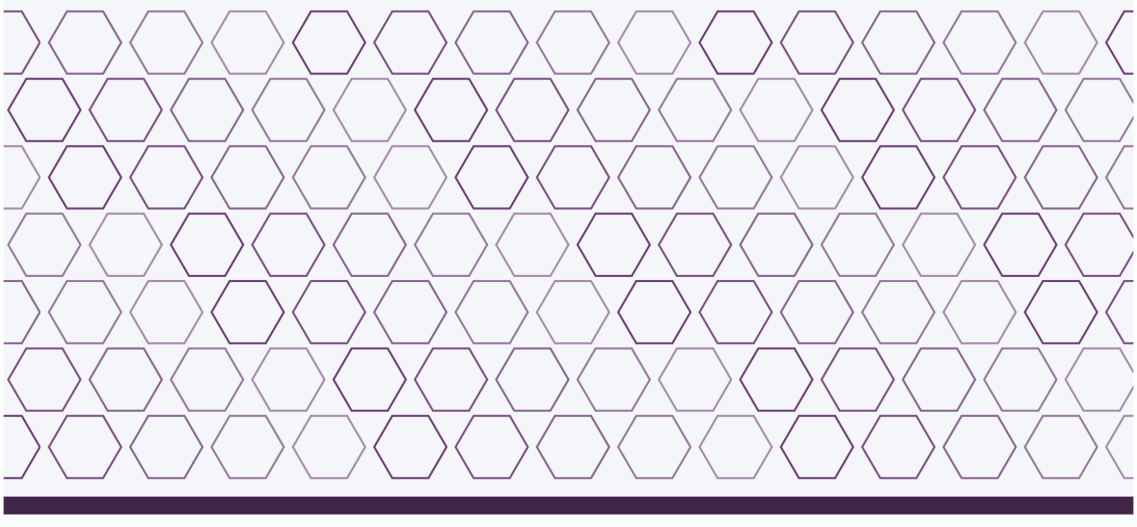
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Cognitive Therapy with Children and Adolescents**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Cognitive Therapy with Children and Adolescents to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 18

# Rational Emotive Behavior Therapy

Ellis's ABC model, the disputing process, and rational emotional flexibility in the counseling room.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the development of rational-emotive behaviour therapy in Albert Ellis's work and the changes it has undergone, including the shift from rational-emotive psychotherapy to rational-emotive behaviour therapy.
- State the ABC model and apply it to a case.
- Explain the concepts of irrational beliefs, disputation, the alternative, and the effective philosophy, and use the language accurately.
- Distinguish the healthy and unhealthy emotions, and explain the clinical and ethical significance of the distinction.
- Describe the three forms of disputation and the five steps of the therapeutic process.
- Apply rational-emotive behaviour therapy to work with children and adolescents, and specify the developmental modifications.
- Evaluate the evidence for the therapy and for its distinctive components.
- Critically evaluate the theory, including its philosophical commitments, its account of emotion, and its cultural assumptions.

## Chapter Overview

Albert Ellis began rational-emotive therapy in the 1950s, after a decade of practice as a psychotherapist and after reading the Stoic philosophers, whose proposition that human distress is caused not by events but by judgments about events struck him as both obviously true and practically useful. He developed the ABC model, which is one of the most widely used explanatory frameworks in the profession, and he remained for forty years its most energetic advocate and its most persistent critic of everyone who took the cognitive model in a more moderate direction.

The therapy's distinctive features are the **dispute** — a structured, argumentative examination of the young person's irrational belief, conducted in a way that is explicitly non-blaming because the belief is understood as a self-defeating strategy that the young person has learned and can therefore unlearn — and the **effective philosophy**, the set of statements that the young person is helped to construct and to live by.

The theory is of particular interest to a student in a course on children and adolescents because of a tension at its centre that Ellis himself foregrounded. The therapy asserts that human beings are responsible for their emotional disturbances, and this is its great pedagogical strength, because it gives a young person genuine agency in the face of circumstances that are frequently outside their control. It is also, in the eyes of many critics, its great liability, because the assertion can be heard as an accusation, and because the therapy's account of what is outside a young person's control is narrower than it should be. This chapter presents the theory in its full strength, describes the modifications required for developmental work, and examines the criticism with the seriousness it deserves.

## 1. Historical Context

### 1.1 Ellis's development

Albert Ellis's path to rational-emotive therapy was distinctive. He was, in succession, a business executive, a teacher, and a psychotherapist, and he had no formal clinical training until later in his career. He was, in his own frequently polemical account, a failed marriage and a mediocre therapist, and he turned to the Stoics, to Descartes, and to the sceptical philosophers in order to make sense of his own difficulty.

In 1955, at a meeting of the American Association for the Advancement of Psychotherapy, Ellis gave a paper that is generally taken as the founding of the therapy, and the paper — on the psychological techniques of getting a new idea and throwing away an old one — was poorly received. The title of the paper is itself a statement of method, and the reception was a fair indication of how radical the proposal was. Rational-emotive therapy was in effect an argument that a human being's characteristic psychological disturbance is not caused by external events, by a past, or by an unconscious, but by the person's own self-defeating beliefs, and that the therapeutic task is to argue the young person out of those beliefs.

### 1.2 The changes

The therapy has changed substantially in sixty years, and a student should know the trajectory, because the popular version is the 1950s version and the practice has moved a long way from it.

**From rational-emotive psychotherapy to rational-emotive behaviour therapy.** The renaming in the 1980s reflected the recognition that the therapy had always included behavioural and experimental methods and that the "rational" label had drawn attention away from them. The modern therapy emphasises the behavioural experiments, the homework, and the emotional and behavioural change, and it is more active and more directive than the popular image of a Socratic argument.

**The move from the exclusive to the inclusive philosophy.** Ellis's original formulation proposed a "philosophy of rational emotions" that was a demanding personal philosophy to be adopted wholesale. The contemporary formulation is the **effective philosophy**, which is more modest: a set of preferences rather than a set of truths, a way of living that is preferable rather than correct, and a set of statements that the young person is helped to construct for their own circumstances. The shift has been important, and it has reduced the theory's metaphysical exposure considerably.

**The development of the psychoeducational and popular forms.** The 1960s and 1970s saw the development of the popular rational-emotive pamphlets, the encounter with the marathon groups, and the formation of the Albert Ellis Institute. The popular form is still widely used in educational and workplace settings and a student should know it, because it is frequently the version with which a young person will have had contact.

**The influence on other theories.** The REBT account of the structure of the irrational belief, the ABC model, the Socratic disputation, the homework, and the concept of the self-disturbance have been absorbed into the cognitive therapies, and several of the most widely used cognitive techniques were developed independently by people working in the Ellis tradition.

## 2. The ABC Model

### 2.1 The structure

The ABC model is a simple sequence.

**A — the activating event.** Something happens. This is the crucial theoretical claim: the event is not the cause of the emotional disturbance, and the "if... then" propositions of the behavioural theorists, which made the event the cause and the behaviour the consequence, are precisely what the theory attacks.

**B — the belief.** The activating event is interpreted, and the interpretation generates the emotional and behavioural consequences. The critical point is that the belief is not the activating event but the person's *interpretation* of it, and that the interpretation is available to awareness and to examination.

**C — the emotional and behavioural consequence.** The emotional and behavioural consequences, which are unhealthy or irrational, follow from the belief rather than from the event.

**A, B, C — the dispute.** The therapeutic intervention, in which the practitioner and the young person examine the belief.

The model's power is its clarity, and the clarity is genuinely useful. A young person who can be helped to see that the "activating event" was a mark of 62 per cent and the belief was "I am a failure" has made a large step, and the step is one that can be made developmentally.

### 2.2 A worked example

Take a sixteen-year-old, Nadia, who has received a text from a friend saying that the friend cannot come to her birthday party. The activating event: the text. The irrational belief: "I am not important enough for people to want to come." The healthy emotion: disappointment, and a proportionate sadness. The unhealthy emotion: shame, and the belief that she is a failure. The behavioural consequence: she does not go to the party, does not tell anyone, and spends the evening in her room.

Ellis's own way of putting it was: "It is not the party that is the problem; it is your belief about the party." The therapeutic work is then to dispute the belief.

## 3. Irrational Beliefs

### 3.1 The definition

An **irrational belief** is, in the modern formulation, a belief that is **inflexible, absolutist, and self-defeating**. The three criteria are: it demands that something must be exactly a certain way; it is a demand, an should, a must, an utterly, an always, a never; and it produces emotional disturbance in the person who holds it, and typically damages the person's relationships and opportunities.

The formulation is important because it is behavioural and functional rather than moral. A belief is irrational not because its content is false but because holding it rigidly is bad for the person holding it. This is a considerably more defensible claim than the one that irrational beliefs are simply "wrong."

### 3.2 The ten or eleven unreasonable beliefs

Ellis's lists of unreasonable beliefs are the most widely known part of the theory, and they are the most frequently trivialised. The classic list is:

1. The misfortune of other people is my fault.
2. I am what my experiences have made me.
3. Something outside of my control must be the cause of my unhappiness.

4. I am what my experiences have made me.
5. My past is the cause of my present behaviour, so I cannot change.
6. It is easier to be sick than to take on the difficulties of change.
7. It is easier to avoid than to face.
8. Life has to be perfect, or it is awful.
9. I must be perfectly good or I will be condemned.
10. I must get what I want, or I will be miserable.
11. It is my fault if I am not happy.

Modern versions have modified the list considerably — the "musts" of the contemporary formulation are about the self ("I must be loved," "I must be competent"), and the emphasis has shifted from a catalogue to a structure. A student should learn the structure rather than the list.

## 3.3 The structure of the belief

The contemporary formulation identifies four elements in the structure of a self-defeating belief: the **triggering event** (the "A"), the **demand** ("I must," "I should," "I have to"), the **belief about the demand** ("If I must do this, then I am a failure" / "If I must not do this, then I am a bad person"), and the **self-disturbance** (the emotional and behavioural consequence).

The **self-disturbance** is the term Ellis used for the unhealthy emotion, and the word is chosen to make the point that the person is disturbing himself. The contemporary practice is to use it as the name for the point at which the intervention is directed.

## 3.4 The dispute

The **dispute** is the core of the therapy, and it is the element most often misdescribed as confrontational. Ellis was explicit that the dispute is not an argument, and the principles that govern it are:

**The dispute is aimed at the belief, not the person.** The practitioner does not say the young person is wrong; the practitioner says the belief is self-defeating.

**The dispute is Socratic.** The practitioner asks questions that expose the belief's inflexibility and its consequences, and does not deliver the alternative.

**The dispute is empirically grounded.** The practitioner asks what evidence the young person has for the belief, what evidence there is against it, and what the practical consequences of holding it are. Ellis insisted that the dispute should not be a philosophical debate about the nature of reality but an examination of the practical consequences of the belief.

**The dispute is unconditional and unconditional in its acceptance of the person.** The young person is accepted whatever the belief.

**The dispute is aimed at the demand.** The most common and most effective target is the demand within the belief — the "must" and the "should" — and the goal is to replace the demand with a preference.

## 3.5 The alternative

The **alternative** is the statement the young person constructs in the place of the irrational belief, and the discipline here is important: the alternative is not the positive opposite of the irrational belief, and it is not the objectively correct statement. The alternative is a **better**, more flexible, less self-defeating statement that the young person can believe and act on.

This is the point at which the therapy is most often misunderstood as a demand for positivity, and the practitioner should be able to give a clear example. The irrational belief "If I am not liked, I am worthless" does not have the alternative "If I am not liked, I am wonderful." The alternative is something like: "Not being liked by everyone is disappointing, and I can cope with that, and I still want to be with these people even if some of them are not available." The alternative is functional, not flattering.

# 4. The Healthy and Unhealthy Emotions

## 4.1 The distinction

The theory distinguishes the **unhealthy emotions**, which are generated by the self-defeating beliefs and are self-defeating in turn — the intense, blaming, absolutist states such as shame, guilt, depression, anxiety, and rage — from the **healthy emotions**, which are generated by the flexible beliefs and which are appropriate responses to a difficult situation. A young person who is disappointed because a friend has not come to a party has a healthy emotion; a young person who is crushed, humiliated, and convinced they are worthless has an unhealthy one.

The distinction is not a claim that some feelings are good and others bad. It is a claim about the *function* of the emotion: the healthy emotions are the appropriate responses to a reality that is genuinely difficult, and they are useful; the unhealthy emotions are the responses to a belief that is not supported by the reality and that damages the person.

## 4.2 The clinical and ethical significance

The significance is large, and it is the aspect of the theory that a student should most carefully understand.

**The clinical significance** is that the unhealthy emotion is the target and the healthy emotion is the goal, which means that the therapy does not aim to remove the young person's distress by reassuring them that everything is fine. It aims to remove the *irrational* component of the distress, which is the belief, and to leave the appropriate response to the actual difficulty. A young person who is disappointed about the party should be disappointed; the work is with the shame and the worthlessness.

**The ethical significance** is more complicated, and the complication is where the theory's central vulnerability lies. The theory asserts that human beings are responsible for their emotional disturbances. Applied to a young person, this assertion can become a claim that the young person is responsible for their depression, which is both false and damaging. The REBT literature has been comparatively attentive to this risk, and the modern formulation is more careful than the popular version, but the risk is real and a practitioner must guard against it.

The guard is the same one that guards the cognitive model: the dispute is conducted against the belief and never against the person, and the examination of the belief is always traced back to its origins and to its function in the young person's life. A dispute that ends with the young person feeling foolish has gone wrong, however technically correct the practitioner has been.

# 5. The Therapeutic Process

## 5.1 The five steps

**Step 1: The relationship.** The practitioner establishes a relationship in which the young person can consider a challenge to their belief without feeling unsafe. Ellis insisted that this is the most important step and the one most neglected, and he was right: the dispute does not work with a young person who does not trust the person doing it.

**Step 2: The diagnosis of the irrational beliefs.** The practitioner and the young person identify the specific irrational beliefs, the demands within them, and the self-disturbances they generate. The aim is to find the beliefs that are actually operating, which is often not what the young person presents.

**Step 3: The learning of the alternative.** The practitioner and the young person work to develop a more flexible, less self-defeating belief, and the young person is helped to see what difference it makes in practice. The "emotional homework" — deliberately provoking the self-disturbance in a controlled situation so that the young person can observe it — is the characteristic technique at this step.

**Step 4: The solution.** The young person is helped to identify the concrete problems in their life and to plan specific action. The theory is emphatically practical, and Ellis objected strongly to therapies in which the young person felt better but continued to behave in the same self-defeating ways.

**Step 5: the prevention of relapse.** The young person is helped to establish a pattern of self-disputing, so that they can catch the irrational beliefs in operation in future.

## 5.2 The emotional homework

The **emotional homework** is the technique most associated with the therapy and it is distinctive. The young person is asked to identify the situation in which the self-disturbance occurs, to deliberately provoke it — by imagining the worst, or by dwelling on the event, or by rehearsing the belief — and then to observe the emotional disturbance as it arises, and to observe it as it changes.

The mechanism is that the young person experiences the disturbance in a controlled setting, learns that it rises and falls, and discovers that it is not the catastrophe that was feared. It is a behavioural and physiological technique, and the cognitive and behavioural elements are both present. With a young person, the technique is modified to be shorter, more bounded, and to be concluded deliberately with a return to the effective philosophy, and it should never be used with a young person whose disturbance is a serious symptom of a mental health condition without a full assessment.

## 5.3 Other techniques

**Rational emotive imagery.** The young person imagines the worst that could happen and then the worst after that, and continues in a chain, and then — the key step — imagines how they could cope with it, and then how they could improve the situation. The technique is highly structured and is one of the more effective ways of working with catastrophising, and it is directly adaptable to a young person who is a catastrophic thinker, which is very common in anxiety.

**The decision-making and the preference.** The therapy works with the young person's decisions and their "shoulds" about others, and the contemporary work on decision-making focuses on the young person's tendency to assign blame and to build a self-image out of the blame.

**The philosophy of the self.** The contemporary formulation uses the "philosophy of the self" — a short personal statement of the young person's core beliefs about themselves, built in the session — and the work on shame, self-forgiveness, and unconditional positive regard for oneself.

# 6. Rational Emotive Behaviour Therapy with Children and Adolescents

## 6.1 The developmental fit

REBT has a specific and unusual claim to relevance with young people, and it is worth making the claim precisely: **the therapy gives a young person a way to be responsible for their own reactions without being responsible for their circumstances.** That is a developmental achievement of some importance, because the central problem of adolescence is precisely the collision between the young person's growing need for agency and the adult world's insistence on their powerlessness.

The therapy also provides a vocabulary that is usable developmentally, because the irrational belief and the demand are concrete and teachable, and because the emotional homework is a behavioural technique that does not require much verbal sophistication.

## 6.2 The developmental modifications

**The language.** The formulation must be translated into the young person's own terms. "The activating event" becomes "what happened"; "the demand" becomes "the rule you are living by" or "the thing you are telling yourself you have to do"; "the self-disturbance" becomes "the feeling that makes it worse." Ellis himself insisted on plain language, and the practitioner should follow the principle with more rigour than the theory's own literature does.

**The scope of the dispute.** With a young person, the dispute should be conducted with the "must" as the target and with the person protected. A useful developmental rule is: dispute the demand, never the person, and never the young person's right to feel what they feel. The dispute is about the inflexibility and the consequences of a rule, not about the appropriateness of the feeling.

**The social context.** The developmental modification with the greatest consequences is the requirement to examine whether the irrational belief is in fact irrational *in the young person's circumstances*. A young person who believes "I must be liked by everyone in order to be safe" may be living in an environment in which the threat of rejection is real and the stakes are high, and disputing that belief with the abstract proposition that "not everyone liking you is terrible" is a callous response to a real situation. The REBT framework itself provides the solution — the "healthy, real, logical, and philosophical" consideration of the actual situation — and a practitioner who omits it has misunderstood the theory.

**The emotional homework with young people.** The homework should be shorter, more bounded, and more frequent, and it should be done with a specific, concrete trigger. And it should never be used to provoke a young person's trauma response, and a practitioner must be alert to the possibility that the homework has triggered something that needs the protocols of Part VI.

**The parent.** The work with parents is often the most valuable component, because parents' own irrational beliefs about their young people are frequently at the origin of the young person's.

### 6.3 The specific risk

The specific risk with young people is the misapplication of the responsibility claim, and the practitioner should be able to describe how to avoid it.

**The avoidance.** The responsibility claim is most safely conveyed by a specific reframe: "Some of what is happening to you is not your fault, and some of how you are handling it is worth working on. Both of those are true, and neither cancels the other."

**The relational basis.** The dispute is most likely to be experienced as an accusation when the young person has a history of being blamed, and the practitioner should establish the relational conditions before the dispute. Ellis's own emphasis on the relationship as the first step was, in retrospect, ahead of its time.

**The pathology risk.** A young person with a diagnosis — of depression, of an eating disorder, of a psychotic condition, of ADHD — may have difficulties that the responsibility claim does not touch, and the dispute should be conducted with the diagnosis in view, not around it.

## 7. Modern Developments

**The emphasis on the self-disturbance and the emotion.** The contemporary therapy has moved closer to the emotion-focused and acceptance-based approaches, with a specific practical point: the distinction between the healthy and the unhealthy emotion is retained, but the theory's earlier tendency to treat the healthy emotion as a goal has given way to a more accurate position in which the goal is the reduction of the self-disturbance and the development of the young person's capacity to experience and to tolerate the healthy emotion.

**The integration with the cognitive-behavioural tradition.** The REBT and CBT traditions have converged substantially, and the differences are now largely matters of emphasis. REBT is more active and more disputational, more philosophical in its commitments, and more focused on the structure of the belief than on its content.

**The applications in education and organisations.** The popular and educational forms of REBT are widely used in schools, in universities, in the workplace, and in sport, and the "rational emotive behaviour education" programmes are among the more empirically supported school-based prevention and promotion programmes.

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## Further Reading for This Chapter

- Ellis, A. (1962). *Reason and emotion in psychotherapy*. — The founding text, and the source of the ABC model.
- Ellis, A., & MacLeod, C. (2011). *Overcoming resistance: A rational emotive behaviour therapy approach*. — The modern practice, and the best single account of the dispute.
- Ellis, A., & Hanson, R. (2007). *Rational emotive behavior therapy: It works for me*. — The practitioner's own account, with the chapters on the theory's development.
- Bernard, M. E., & DiMaria, M. J. (Eds.). (2011). *Rational emotive behavior therapy: It works for me*. — A more substantial account of the contemporary theory and practice.

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## 8. Case Illustration: The Young Person Who Was Responsible for Everything

**The referral.** Oskar, sixteen, is referred by a school for "anger, defiance, and low mood." The school reports that Oskar has had four suspensions in a term, that he has been in a fight, and that he is failing two subjects. The school counsellor notes: "Oskar will not accept that he is in the wrong. He argues with everyone. His mother says he is exactly like his father, who left when Oskar was six."

**The first session.** The practitioner begins the REBT assessment in the standard way: what happened, what did you think, how did you feel. Oskar is a good reporter, articulate, and clearly accustomed to being asked what he thinks.

Oskar describes being suspended after refusing to hand in a piece of homework that was, he says, "pointless, because the teacher had already told me I'd fail it." The activating event: the teacher's remark. The belief: "If I fail it, everyone will know I'm stupid, and there's no point in doing anything." The self-disturbance: a hot anger that came out as shouting at the teacher, and a shame and hopelessness that came out as not caring.

**The dispute.** The practitioner does not dispute the belief directly, and the sequence is worth describing because it is a good example of the technique with a young person.

**First, the acknowledgement.** "That sounds genuinely unfair. He had decided you were going to fail before you'd done the work. That's a rotten thing to have done."

**Then, the question about the evidence.** "So what actually happened when you handed it in?" Oskar says: "I didn't hand it in. I refused." "Ah. So you don't know what the mark would have been?" He says: "No." "And you were sent out of the room?" He says: "Yes." "So you didn't find out the mark, and you found out you were in trouble. And you'd already worked out that you were going to fail."

The dispute is not directed at the belief that the teacher was unfair — the practitioner has just affirmed it. It is directed at the *inference* from that unfairness to "everyone will know I'm stupid and there's no point in anything."

**Then, the question about the demand.** "There's a 'no point' in there. Tell me about the rule that's running." Oskar is quiet, and then: "If you can't do it, you might as well not bother." The practitioner asks what that rule is for, and Oskar says: "My dad used to say that. He said if you can't do something you're not going to be anything, so don't bother trying." And then, after a pause, in a much quieter voice: "I can't do anything anyway."

**The developmental work.** The practitioner does not go further into the father in the first session. What she does is move to the second part of the dispute, which is the point at which the theory's own structure is most useful with a young person: the examination of the actual situation, and the construction of an alternative that is *not* the positive opposite of the belief.

"There are two things there, Oskar. One is that the teacher did treat you badly, and that's real, and I'm not going to pretend it isn't. The other is the rule: 'if you can't do it, you might as well not bother.' That rule was

your dad's rule, and it was a rule that worked in his life. Does it work in yours? If you'd handed it in and got a bad mark, what would actually have happened?"

Oskar thinks about it, and this is the first genuine shift in the session. "Nothing. I'd have got a bad mark and a bit of embarrassment and it'd have been over." The practitioner says: "So the rule is protecting you from a small thing, and costing you a big thing. That's the trade."

**The alternative.** "Can you think of a rule that would work better?" Oskar tries. "I don't know." "How about: 'if I'm bad at something, that doesn't mean I'm stupid, and I can still be good at other things, and there's no point in deciding in advance that it's not worth trying'?" He says, after a while: "That's a lot." "It is. Do you want to take part of it? Just the bit about the deciding in advance?"

**The homework.** The homework is deliberately small and behavioural: hand in the next piece of homework even if it's bad, and write down the two thoughts that come up before and after, and rate the anger. The emotional homework — deliberately provoking the shame — is postponed, and the practitioner explains why: "We're going to do that, and it's going to be useful, but not this week. I want to do it when I know you'll not be in a bad way afterwards."

**The work with the mother.** The mother is invited to attend, and the practitioner works with her on her own irrational beliefs, which are extensive. The central one is: "He is exactly like his father, and he'll end up exactly like his father." The dispute with the mother is the same as the dispute with the young person, and it is the same in structure: the belief is a prediction, the prediction is a self-fulfilling prophecy, and the alternative is behavioural, not rhetorical — she can treat Oskar as a separate person from his father, and can begin to do so in one specific situation, and can observe what happens.

**The outcome.** Over twelve sessions, Oskar's suspensions reduce to none, his second-set work improves, and the anger reduces in frequency and in intensity. The conflict with the teacher is addressed directly, in a meeting, with Oskar's consent, and the practitioner is present. The father is not contacted; the practitioner discusses with Oskar why, and Oskar says "he'd just say I was being soft." The core belief — "if you can't do it, you might as well not bother" — is identified, and Oskar can quote it and question it, which is the practical indicator of the change the therapy was aiming at.

**The teaching point.** The dispute worked because it was aimed at the *demand* and the *inference*, not at the young person's complaint, which was legitimate and was affirmed. And the work with the mother was not an optional addition; the mother's belief was one of the two sources of the belief, and the mother's change was what allowed the young person's change to hold.

## 9. Evidence and Research Support

**The evidence for rational-emotive behaviour therapy is stronger than the popular image of the theory suggests, and it is stronger in the educational applications than in the clinical ones.**

**Individual therapy.** A number of controlled outcome studies, mostly from the 1990s and 2000s, have reported favourable outcomes for REBT with adults in the treatment of anxiety, depression, and general emotional disturbance, frequently with large effects in comparison to wait-list controls and comparable effects to other bona fide treatments. A meta-analytic review reported a moderate-to-large pooled effect across studies. The methodological quality of the studies is variable, and a substantial number of them were conducted by investigators with an allegiance to the theory.

**Educational and organisational applications.** The "rational emotive behaviour education" programmes and the workplace programmes have a larger and generally better-evaluated evidence base than the clinical literature, with meta-analyses reporting small-to-moderate effects on students' mental health, on their irrational beliefs, and on their academic and social outcomes. These programmes are among the more defensible school-based mental health interventions available, and they are directly relevant to a student in this course who may work in a school.

**Group and community applications.** The theory's group and community variants have been evaluated in a number of trials, and the popular and semi-popular formats have a substantial record of use, with the standard caveats about fidelity.

**The component evidence.** The emotional homework and the rational emotive imagery have been evaluated as components in a small number of studies, with positive findings, and the dispute technique has been compared with more purely cognitive techniques in a small number of studies with comparable outcomes.

**The limits.** The evidence base is dominated by adult populations, by individual therapy, and by studies with allegiance. The evidence for the developmental adaptations with children and adolescents is very thin, and the developmental applications are widely described in the clinical literature and poorly evaluated. The popular and educational forms should not be confused with the clinical therapy, and their evidence base should not be transferred to it.

## 10. Strengths and Limitations of Rational Emotive Behaviour Therapy

**Strengths.** The ABC model is one of the most useful explanatory frameworks in the profession, and its clarity makes it directly teachable to a young person. The theory gives a young person genuine agency, which is developmentally valuable and often missing from the other approaches. The responsibility claim can be made in a form that is careful and true, and that form is worth teaching. The dispute is a well-specified, learnable technique with a clear mechanism. The therapy is practical, with homework and specific action, and the emphasis on the solution and on relapse prevention is more developed than in most humanistic therapies. The educational applications are among the more defensible school-based interventions available.

**Limitations.** The responsibility claim is the theory's central liability, and in the hands of an inexperienced practitioner it becomes a claim that the young person is to blame for their difficulties. The theory's account of what is outside the young person's control is too narrow, and the remedy for this is built into the framework (the "healthy, real, logical and philosophical" consideration of the actual situation) and is often omitted. The dispute is easy to conduct badly, and the theory's own literature contains more than a few examples of practitioners who were intellectually impressive and interpersonally clumsy. The developmental evidence is thin. And the theory's metaphysical commitments — about human nature, about rationality, and about the good life — are more assertive than its empirical warrant, and the contemporary shift to the "effective philosophy" has reduced but not eliminated the exposure.

## 11. Cultural and Ethical Considerations

**Culture and the responsibility claim.** The theory's anthropology is unusually explicit and unusually contestable. Ellis's position is that human beings are rational, that they disturb themselves unnecessarily,

and that the most fully human course is the acceptance of a philosophy that minimises self-defeat. This is a recognisably Western, individualist, rationalist, and Enlightenment-derived account, and the cross-cultural literature finds substantial variation in the endorsed value structures, including a strong emphasis in many non-Western traditions on relational, contextual, and duty-based reasoning.

Three practical implications follow for a practitioner working with a young person from a different cultural framework.

**The young person's "irrational" belief may be a rational response to a real environment.** A young person who believes that "I must not show weakness in front of my family" may be operating in a context in which that is entirely appropriate. Disputing it in the abstract will be both inaccurate and disrespectful.

**The theory's own framework accommodates the cultural consideration.** The demand for the *practical* consideration of the actual situation, and the emphasis on the young person's own effective philosophy, means that the theory can be used to examine the young person's demands in the light of their own context rather than in the light of the practitioner's.

**The "shoulds" of the family.** In a family in which the young person's obligations are genuinely heavy, the irrational beliefs may be located in the *family's* demands rather than in the young person's, and the work is then with the family. This is a point at which the REBT practitioner needs the systemic perspective of Part V.

**The ethical question of the dispute.** Two specific requirements follow. First, the dispute must never be a substitute for assessment: a young person whose distress is a symptom of a clinical condition — depression, an eating disorder, PTSD, a psychotic condition, ADHD, or the consequences of abuse — must be identified as such, and the dispute conducted within the treatment for that condition rather than instead of it. The theory's own literature is clear on this point, and the practice is not always careful. Second, the responsibility claim must be stated in the careful form, and the practitioner must be prepared to say to a young person: "Some of this is not your fault, and some of it is worth working on."

**The ethical value of the theory.** The ethical value of the theory is also considerable and should be recorded. It is one of the few approaches in the field that treats a young person's own thinking as the place where change can occur, that does not require the young person to be passive, and that gives them a way of relating to their own difficulties that is dignified. For a young person who has been told that they are the problem and that the problem is in their head, a therapy that says "you are the one who has been studying this problem, and there is a better way of running your life than the one you are running" is a considerable gift.

## 12. Key Terms

### Key Terms at a Glance

- **Rational-emotive behaviour therapy** — A therapy developed by Albert Ellis in which psychological disturbance is attributed to self-defeating beliefs about oneself, and in which the therapeutic task is to dispute those beliefs and to construct more flexible alternatives.
- **ABC model** — The framework in which the activating event (A) is interpreted by a belief (B) which generates the emotional and behavioural consequences (C), with the therapeutic intervention designated ABC.
- **Activating event** — The occurrence to which the young person attributes their emotional disturbance; in this theory, a cause rather than a sufficient explanation.
- **Irrational belief** — A belief that is inflexible, absolutist, and self-defeating; functional rather than moral in definition.
- **The unreasonable beliefs** — Ellis's catalogue of characteristic self-defeating propositions, most of which concern demands and shoulds.
- **Demand** — The "must," "should," or "ought" within a belief; the contemporary primary target of the intervention.
- **Self-disturbance** — Ellis's term for the unhealthy emotional and behavioural consequence generated by a self-defeating belief.
- **Healthy emotion** — An appropriate emotional response to a genuinely difficult situation, generated by flexible beliefs; the goal of the therapy rather than its removal.
- **The dispute** — The structured examination of a belief in which the practitioner argues against its inflexibility and its consequences rather than against its content, and which is Socratic, empirically grounded, and aimed at the belief and not the person.
- **The alternative** — A more flexible, less self-defeating statement that the young person constructs in place of the irrational belief; functional rather than flattering.
- **Effective philosophy** — The contemporary formulation in which the philosophy is a set of preferences rather than a set of truths, and which the young person constructs for their own circumstances.
- **Emotional homework** — The deliberate provocation of the self-disturbance in a controlled setting so that the young person can observe it rise and fall.
- **Rational emotive imagery** — The technique of imagining the worst repeatedly and then of coping with and improving it.
- **Philosophy of the self** — A short personal statement of the young person's core self-beliefs, constructed in the session.

**Key Terms at a Glance (continued)**

- **Unconditional positive regard for oneself** — A component of the contemporary effective philosophy, and the direct answer to the shame-based core beliefs that the theory treats most often.
- **The four ways of thinking** — The contemporary formulation of the structure of a self-defeating belief: the triggering event, the demand, the belief about the demand, and the self-disturbance.
- **Self-disputing** — The skill of catching one's own irrational beliefs in operation; the relapse prevention component of the therapy.
- **REBT education** — The school and workplace applications of the theory, with a better-developed evidence base than the clinical applications.
- **Emotional and behavioural consequences** — The full C of the ABC model, comprising the emotional, the behavioural, and the physiological.

## 13. Chapter Summary

Rational-emotive behaviour therapy was developed by Albert Ellis from the 1950s, drawing on the Stoic proposition that human distress is caused not by events but by judgments about events. It has changed substantially: the renaming from rational-emotive psychotherapy in the 1980s, the shift from the exclusive philosophy to the more modest effective philosophy, the development of the popular and educational forms, and the absorption of its methods into the wider cognitive tradition.

The ABC model is the framework: the activating event is interpreted by a belief, which generates the emotional and behavioural consequences, and the intervention is the dispute. Irrational beliefs are defined functionally as inflexible, absolutist, and self-defeating, and the contemporary formulation targets the demand within the belief and the self-disturbance it generates. The dispute is Socratic, empirically grounded, aimed at the belief and never the person, and concerned with the practical consequences of holding the belief; the alternative it produces is functional rather than flattering.

The healthy/unhealthy emotion distinction is clinically and ethically significant: the target is the self-disturbance, the goal is the healthy response to a real difficulty, and the therapy does not consist of reassuring a young person that everything is fine.

The developmental fit is unusual and valuable, because the therapy gives a young person agency without implying responsibility for their circumstances — provided the responsibility claim is stated carefully, and provided the framework's own requirement that the actual situation be taken into account is observed. The case of Oskar shows the dispute working at its best, aimed at the demand and the inference rather than at the young person's legitimate complaint, and it shows the work with the mother as the component that made the young person's change hold.

The evidence is stronger in the educational applications than in the clinical ones, is dominated by adults and by investigators with allegiance, and is very thin for the developmental adaptations. The theory's central liability is its responsibility claim, and the ethical practice of the theory depends on holding that claim in the careful form.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Trace the development of rational-emotive therapy from 1955 to the present and state the three principal changes.
2. State the ABC model and apply it to a case of your own.
3. Define an irrational belief in functional terms, and explain why the functional definition is preferable to a truth definition.
4. What is the demand, and why is it the contemporary primary target of the intervention?
5. Explain the dispute, state its five governing principles, and distinguish it from an argument.
6. What is the alternative, and why is the positive opposite of an irrational belief not an acceptable alternative?
7. Distinguish the healthy from the unhealthy emotion, and state the clinical and ethical significance of the distinction.
8. Describe the emotional homework and the rational emotive imagery, and explain the mechanism of each.
9. What are the developmental modifications required for rational-emotive behaviour therapy with a young person?
10. State the specific risk of the responsibility claim in work with young people, and state the form of the claim that is safe.

## 15. Practice Activities

**Practice and Application**

### Activity 18.1 — The Demand Audit (90 minutes, individual, written)

Take a difficult situation from your own recent life and write out the belief you held in the moment, in the four-part structure: the triggering event, the demand, the belief about the demand, and the self-disturbance. Then write the alternative — a functional statement, not a positive one. Then write one specific thing you would do differently this week. Bring the three written items to the seminar. The exercise is the therapy in miniature and it is more revealing than most students expect.

### Activity 18.2 — The Dispute Rehearsal (2 hours, triads)

One student presents a real irrational belief; one disputes it, following the five principles; one observes, specifically for whether the dispute ever slid into telling the young person they were wrong about their feeling rather than about their belief. The observer also notes the moment at which the young person became willing to consider the alternative. That moment is the thing to learn to recognise, and it is not the same as compliance.

### Activity 18.3 — The Cultural Context of a Demand (75 minutes, pairs)

Take one of the ten unreasonable beliefs and write it in two registers: the register of a young person in a Western individualist context, and the register of a young person in a family that emphasises duty, obligation, and continuity across generations. Then write what the demand would look like in each case, and what would happen if the practitioner disputed the first in front of the second family. The exercise is the most direct preparation for the cultural work in Chapter 48.

## 16. Annotated Reading Pathways

**For the theory.** Read Ellis (1962) for the founding statement and Ellis and MacLeod (2011) for the contemporary practice. The contrast between the two is instructive, and the popular pamphlets, which are short and easily obtained, are also worth reading precisely as a historical document.

**For the developmental work.** Read the literature on the educational applications, and then the clinical developmental literature with attention to what has been adapted and what has been dropped.

**For the evidence.** Read the meta-analytic reviews of the clinical outcome studies and of the educational programmes separately, and note the difference in the quality and quantity of the evidence.

**For the critique.** Read the cross-cultural literature on moral reasoning and on the value structures that inform implicit theories of the good life. The theory's account of human nature is one of the most explicit in the profession and one of the most culturally marked, and a student who can specify the marking is better equipped to use the theory well.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Rational Emotive Behavior Therapy** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is ellis's ABC model, the disputing process, and rational emotional flexibility in the counseling room. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion, behaviour,

skills practice, and relapse prevention. The mechanism to keep in view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Daniel is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Daniel's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Daniel explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Daniel chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Daniel has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Daniel can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Rational Emotive Behavior Therapy

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Rational Emotive Behavior Therapy** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Rational Emotive Behavior Therapy** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

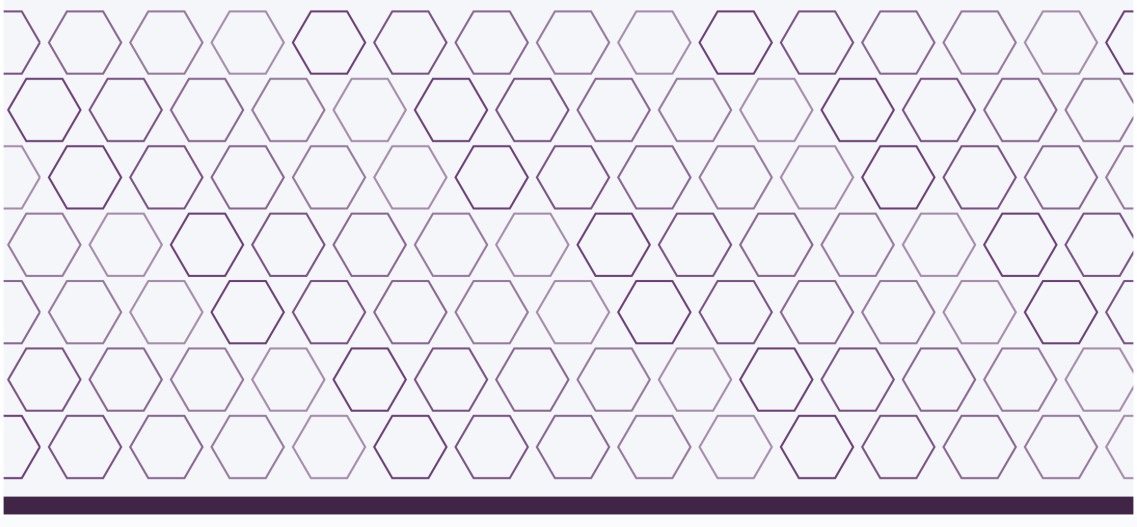
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Rational Emotive Behavior Therapy**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Rational Emotive Behavior Therapy to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 19

# Schema Therapy: Modes, Styles, and Early Maladaptive Schemas

Young's model of personality organisation and the schema-focused treatment of adolescents.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the development of schema therapy, including Young's departure from the cognitive tradition and the relationship to the Beckian research tradition.
- Explain what an early maladaptive schema is, and describe the five domains and the eighteen schemas.
- Explain the three coping styles — surrender, avoidance, and overcompensation — and their relationship to the schemas.
- Describe the mode model, including the child modes, the dysfunctional coping modes, the dysfunctional parent modes, and the Healthy Adult mode.
- Construct a schema and mode map for a case.
- Describe the principal techniques of schema therapy and state their indications and cautions.
- Apply schema thinking to work with children and adolescents, and specify the developmental modifications.
- Evaluate the evidence for schema therapy and for the specific components.

## Chapter Overview

Schema therapy was developed by Jeffrey Young and his colleagues in the 1980s and 1990s, initially as an extension of Beck's cognitive therapy for patients with what Young termed "characterological" problems — pervasive, characterological, and typically developmental difficulties that did not respond to the standard cognitive techniques. Beck himself was for many years an opponent of the new direction, on the ground that the schema model's constructs lacked empirical specificity; the dispute between the two traditions is one of the most instructive in the history of the field, and it has been substantially resolved by subsequent research.

The theory's central claims are that a young person's difficulties are maintained not only by current beliefs but by **early maladaptive schemas** — patterns of experience and affect, concerning the self and others, formed in childhood, and enduring across the lifespan; that the young person responds to these schemas with one of three **coping styles** — surrender, avoidance, or overcompensation — each of which is an attempt to cope with the schema and each of which maintains it; and that the young person's experience is organised in **modes** — distinct psychological states, some of them childlike, some of them "parent-like," and one of them, the Healthy Adult, from which the therapy is conducted.

The theory is of considerable relevance to work with young people, and for a reason that is not obvious at first: it is one of the very few theories in the field that takes seriously the developmental origins of a young person's difficulties, and it does so in a way that is compatible with a developmental practice rather than in opposition to one. A schema is by definition formed early, and a young person with a schema of abandonment is a young person whose developmental task of trust has been obstructed. This gives the therapist a language for the developmental work that is unusually precise.

## 1. Historical Context

### 1.1 Beck's research programme

The single most important development in the psychotherapy research literature in the 1980s was Aaron Beck's programme of psychotherapeutic research, funded by the National Institute of Mental Health, which demonstrated that specific treatments could produce specific, measurable improvements in specific disorders, and which established the methodology — the manualised treatment, the controlled trial, the components analysis, the prediction study — that transformed the profession's claim to scientific status.

The programme produced two of the field's most robust findings, which are also among its most striking. The first is the finding that a substantial proportion of patients with personality disorders, chronic depression, and complex presentations do not respond to standard cognitive therapy. The second is the finding that the *personality* of the patient — its structure, its rigidity, its depth — predicts the outcome of treatment at least as well as the symptom, and that the treatments that work for these patients are those that address the characterological level rather than the symptom level.

## 1.2 Young's departure

Jeffrey Young was a member of Beck's research team, and the second finding above is what made his work possible. Young's observation, developed through the clinical application of Beck's cognitive therapy with difficult patients, was that the standard techniques worked on the surface and left the underlying structure untouched, and that the patients who did not respond had a characteristic profile: they were rigid, they had interpersonal difficulties that the cognitive techniques did not address, they were prone to a recurrence of their difficulties under stress, and their beliefs were deep, unconditional, and global rather than situational.

Young's proposal was that what the standard cognitive model described as "core beliefs" were better understood as **early maladaptive schemas** — patterns of experience and affect, concerning the self and others, formed during childhood and adolescence, and enduring throughout the life span, and functioning as the basis for a wide range of later difficulties. The theory's claim is that the schemas are not merely beliefs but are deeply held **patterns of feeling and thinking about the self and others**, with their own developmental history.

## 1.3 The dispute with Beck

Beck was a long-standing opponent of schema therapy, and the reasons are instructive. Beck's objection was that the schema constructs were not specified with the precision necessary for empirical test, that the term "schema" was used in a way that implied a theoretical construct without an operational definition, and that the theory was in danger of becoming unfalsifiable.

The dispute was eventually resolved empirically, in a series of studies in the 1990s and 2000s that demonstrated the construct validity of the schema constructs, the reliability of the self-report and clinician-rated measures of the constructs, and their predictive validity for treatment outcome. The resolution did not produce a final synthesis of the two traditions, and the two remain distinct in practice, but the outcome of the dispute was a substantial increase in the field's respect for the characterological level.

## 1.4 The expansion of the theory

The theory has been extended in several directions since the 1990s: to the treatment of personality disorders, with the development of the schema-focused treatments for borderline and avoidant personality disorder; to group and to couple therapy; to the developmental and childhood applications, which are the subject of Section 7; and to the emotion-focused and somatic traditions, with which the theory has become

increasingly integrated.

# 2. Early Maladaptive Schemas

## 2.1 The definition

An **early maladaptive schema** is a pattern of experience and affect, concerning the self and others, that is formed in childhood, is deeply felt, and is enduring across the lifespan. The definition has five components, each of which matters.

**"Pattern"** — the schema is a generalisation, not a specific memory. A young person with a schema of abandonment does not have a specific memory of abandonment that is being retrieved; they have a general expectation, built from many instances, that people will leave.

**"Experience and affect"** — the schema includes the emotional quality of the original experience, not only its cognitive content. The schema of abandonment is not only the belief "people will leave"; it is also the feeling of the small child's fear and grief.

**"Concerning the self and others"** — the schema is always about a relation between the self and others.

**"Formed in childhood"** — the developmental origin is essential to the construct, and a schema formed at four and a schema formed at fourteen are different in their implications.

**"Enduring"** — the schema persists across the lifespan and across the life circumstances, which is why it is not adequately addressed by working on current beliefs.

## 2.2 The five domains and the eighteen schemas

The schemas are organised into five domains.

**Disconnection and rejection** — the sense of not belonging, of being fundamentally unacceptable or unwanted. The schemas: Abandonment/Instability; Mistrust/Abuse; Emotional Deprivation; Defectiveness/Shame; Social Isolation/Distrust.

**Impaired Autonomy and Performance** — the sense of being impeded in the ability to act independently, to express one's needs, and to function. The schemas: Dependent/Submissive; Vulnerability to Danger or Illness; Unrelenting Standards; Self-Sacrifice; Approval-Seeking/Recognition.

**Impaired Limits** — the sense of difficulty in setting limits, in protecting oneself, and in tolerating a range of emotions and impulses. The schemas: Entitlement/Grandiosity; Self-Control/Discipline.

**Other-Directedness** — the sense of surrendering control of one's life to others in order to avoid conflict, blame, or abandonment. The schemas: Subjugation/Self-Sacrifice; Self-Approval/Recognition.

**Overvigilance and Inhibition** — the sense of being unable to relax, to express spontaneous emotion, or to meet the world with spontaneous interest. The schemas: Emotional Deprivation; Pessimism/Negativity; Unrelenting Standards; Approval-Seeking/Recognition.

The full list of eighteen schemas with their descriptions and their typical childhood antecedents is reproduced in Appendix C.

## 2.3 The developmental antecedents

Each schema is associated with a set of **typical childhood antecedents** — the environmental conditions in which the schema is likely to have formed, and the unmet emotional needs that it reflects. The schema of Abandonment/Instability, for example, has antecedents including death or loss of a parent, chronic illness in a parent, parental unavailability, and a parent who was inconsistent, and it reflects the unmet need for a secure base and for stability.

The developmental antecedents are the most clinically important part of the schema construct for work with young people, because they tell the practitioner what to look for in the developmental history and they indicate what the young person's unmet developmental need is. A young person with a schema of Defectiveness/Shame and a schema of Unrelenting Standards has a developmental history of criticism and conditional approval, and the unmet need is unconditional acceptance. This is a precise and usable formulation, and it maps directly onto the developmental material in Chapter 9.

# 3. Coping Styles

## 3.1 The three styles

A young person confronted with a schema responds in one of three **coping styles**, and the style is the mechanism by which the schema is maintained.

**Surrender.** The young person allows the schema to be enacted, and behaves in ways that are consistent with it and that confirm it. A young person with a schema of Abandonment who is in a relationship with an unreliable partner will tolerate the unreliability, will not protest, will minimise the harm, and will — when abandoned — experience it as a confirmation rather than as a surprise. Surrender is the mode of a young person who has learned that protesting does not work.

**Avoidance.** The young person avoids situations in which the schema is likely to be activated. A young person with a schema of Defectiveness/Shame will avoid the swimming lesson, the oral presentation, the birthday party, or any situation in which they might be exposed. Avoidance is the mode of a young person who has learned that being seen is dangerous.

**Overcompensation.** The young person does the opposite of the schema, in an exaggerated and rigid way, in order to prevent it from being activated. A young person with a schema of Rejection will become hyper-competent, hyper-controlling, and compulsively successful, and will experience the vulnerability of the schema as intolerable, and the overcompensation will bring its own costs, including the failure to form relationships in which the vulnerability could be tolerated.

## 3.2 The maintenance cycle

The three styles maintain each other in what the theory calls the **schema coping cycle**, and a practitioner should be able to describe it.

The schema is activated. The young person responds with the characteristic style. The style produces a short-term benefit — the anxiety is reduced, the rejection is avoided, the vulnerability is not felt, or the achievement is achieved. The style then produces consequences in the longer term: the interpersonal patterns that confirm the schema, the narrowing of the young person's world, the depletion of their resources, and the reinforcement of the belief that the schema is true.

The clinical implication is that the coping style is where the work is done, and that the schema itself is not the target of intervention in the early phase. A practitioner who attacks the schema directly will be resisted, because the coping style is what is protecting the young person from the schema.

# 4. Modes

## 4.1 The mode model

**Modes** are the distinct psychological states in which a person finds themselves, each with its own characteristic feelings, thoughts, and behaviours, and each organised around a particular need or aim. The mode model is a more immediately usable clinical tool than the schema construct, and it is the level at which most therapists actually work.

**Child modes** are the young, vulnerable, feeling states that are frozen in the child's experience of the original event.

**Vulnerable child modes** are the modes in which the young person feels small, sad, scared, ashamed, or helpless — the emotional core of the schemas, and the mode in which the young person's unmet needs are felt.

**Angry child modes** are the modes in which the young person feels anger, resentment, or defiance, usually in response to a perceived violation of their needs. The angry modes are the emotional complement of the vulnerable modes and are frequently the mode in which a young person presents to services.

**Punitive parent modes** are the internalised representations of the critical, blaming, and demanding authority that the young person has absorbed, and they are the "should" voice of the young person's own inner world. The punitive parent modes are the source of the "I must be perfect" experience and of the harsh self-criticism that is so common in young people with high achievement and with self-harm.

**Demanding parent modes** are the internalised standards that drive the young person to perform, to achieve, and to never be satisfied. Their function is to protect against vulnerability, and their cost is the relentless standards schema and the burnout that follows.

**The healthy adult mode** is the mode from which the therapy is conducted, and it is the mode in which the young person can reflect, plan, and act. The therapeutic task is to strengthen it.

## 4.2 The healthy adult and the child's needs

The theory's account of the therapeutic task is that the healthy adult mode is developed in the context of a relationship with a therapist who provides, in a limited and carefully bounded way, the **reparenting** that the young person did not receive in childhood. The therapist supplies the experience of being met with empathy, with unconditional regard, and with the experience of a limit that is set kindly and consistently, and the young person internalises the experience of the therapist's mode.

This is one of the most contested claims in the theory, and the student should know the history: it derives from the object relations tradition, and the developmental research on attachment and on the internal working models provides a substantial empirical rationale for the claim that early relationships are formative. What the theory adds is the clinical specification of how the formative experience is provided in a bounded and ethically appropriate way, and the limits on the role are set out in Section 10.

## 5. Formulation: The Schema and Mode Map

The schema and mode map is the formulation instrument, and it is one of the more useful in the field.

**The schema map** specifies: the young person's early maladaptive schemas in order of priority; the coping style in relation to each; the specific childhood antecedents; the unmet emotional needs; the typical triggers in the present; the characteristic thoughts, feelings, and behaviours; and the short-term and long-term consequences.

**The mode map** specifies: the child modes and the dysfunctional coping modes in relation to each; the typical triggers for each; the mode in which the young person presents; the mode in which the young person is most vulnerable; the internal dialogue between the modes; and the conditions under which each mode is entered.

The maps are used with the young person, and the practice of drawing them is itself an intervention: a young person who has been in a difficult pattern for years frequently has no idea that they are moving between states, and seeing the map is often the moment at which the pattern becomes tractable.

## 6. The Therapeutic Techniques

### 6.1 Limited reparenting and the empathic confrontation

**Limited reparenting** is the therapeutic relationship itself, and the specification is important: the therapist provides warmth, empathy, understanding, and validation, and the *unconditional positive regard* is the core, but it is provided within clear and consistent limits. The therapist does not permit the young person to abuse, manipulate, or overwhelm; the therapist does not do the young person's homework; the therapist does not function as a parent substitute beyond the boundaries of the relationship. The theory calls this "the perfect parent" model, and the ethical specification of it is set out in Section 10.

**Empathic confrontation** is the technique of bringing the young person's own conflicting positions into direct contact, in a supportive and non-blaming way. "You've said you want to be a doctor, and you've also said you want to be free to choose, and I don't think both of those can be true at the same time, and I wonder which one you want more." The confrontation is empathic in that it names the conflict without judging either side of it, and it is the technique that distinguishes schema therapy from the cognitive therapies in their handling of the deeper material.

### 6.2 Imagery rescripting

**Imagery rescripting** is the technique in which the young person is asked to imagine a scene from the original childhood experience, to identify the mode in which they are then, and to imagine a version of the scene in which the unmet need is met — by the child version of themselves, or by a "good enough" caregiver figure who supplies what was not supplied.

Imagery rescripting is one of the most used techniques in the whole field, and it is one of the most useful with young people, because the child version of the young person is a powerful and accessible image, and because the young person can be helped to see the caregiving they needed and did not receive, which is a developmental fact rather than a cognitive exercise.

The cautions are specific and important: the young person must be in a sufficiently stable state, the imagery must be approached at a pace the young person can tolerate, the work must be done with the mode explicitly

identified, and the technique should not be used with a young person in acute crisis.

### 6.3 Chair work and dialogue work

**Chair work** places the young person in a dialogue with a part of themselves, usually one of the internal modes, seated in an empty chair. The young person speaks from one mode and then from the other, and the aim is to enable a resolution — typically, the young person learning to take the part's perspective and to recognise its need, and the mode then losing some of its grip.

**Dialogue work** is the more structured, more cognitively oriented version, in which the two "voices" are explicitly written out, examined for their evidence, and then restated in a more balanced form. It is more behavioural and more concrete than the chair work, and it is more appropriate with a young person who cannot sustain the affective intensity of the chair work.

### 6.4 The behavioural and cognitive elements

Schema therapy also includes the standard cognitive and behavioural techniques — the thought record, the behavioural experiment, the exposure, the activation of the healthy adult — and their use is differentiated by the mode in which the young person is. The clinical skill is to choose the technique that matches the mode: behavioural activation and problem-solving in the healthy adult mode; the empathic confrontation in the vulnerable child mode; the dialogue work with a demanding parent mode.

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## Further Reading for This Chapter

- Young, J. E. (1990). *Cognitive therapy for depression*. Guilford. — The foundational text and the source of the schema concept.
- Young, J. E., Klosko, J. S., & Weisbarth, J. (2003). *Schema therapy: A practical guide*. Guilford. — The standard clinical text, and the source of the coping styles and the modes.
- Young, J. E., & Klosko, J. S. (2014). *Schema therapy: The art and science of mental health change*. — The mature account.
- Waters, C. A., et al. (2015). *Schema therapy for personality disorders*. — The evidence base in detail.
- Lobban, M. (2018). *A guide to the assessment and treatment of early maladaptive schemas*. — A practical clinical guide.
- Roisman, G. I., & Wall, J. (Eds.). (2017). *The handbook of schema therapy*. — The most comprehensive account.

## 7. Schema Therapy with Children and Adolescents

### 7.1 The developmental fit

The schema theory is developmentally congruent in a way that is unusual among the characterological therapies, and there are three specific reasons.

**The schemas are by definition developmental.** A young person with a schema of Defectiveness/Shame has a schema that was formed when the young person's worth was contingent on performance, and the therapist's task is to identify when and how that happened. The theory's insistence on the childhood antecedents is a developmental insistence, and it means that the therapist is doing developmental work by doing schema work.

**The mode model captures the adolescent's reality.** The adolescent's experience of being in one state and then another — the enthusiastic, engaged, hopeful adolescent on Monday and the ashamed, self-critical, self-hating adolescent on Thursday — is precisely what the mode model describes, and naming it is often the first thing that makes the pattern visible to the young person and to their family.

**The developmental task is explicitly in the model.** The unmet needs that the schemas reflect — for a secure base, for autonomy, for freedom to express emotion, for realistic limits, for acceptance of the self — are the developmental needs, and the theory's reparenting specification is a bounded provision of what was not provided.

### 7.2 The developmental modifications

**The developmental task of the therapeutic relationship.** The adult specification of limited reparenting has to be modified in two ways with a young person. First, the boundary is the boundary: a young person who has been harmed by an adult who was supposed to care for them will test the therapist's reliability, and the therapist's consistency is the treatment. Second, the developmental literature on the adolescent's attachment renegotiation is relevant: the young person is in the process of moving from dependence to autonomy, and a therapist who provides too much warmth and too little challenge will be experienced as another adult to be managed.

**The use of the child version.** The imagery rescripting's use of the "child version" of the young person is developmentally appropriate and powerful, and it requires care. The work is not to regress the young person; it is to give them access to a part of their own experience that has been inaccessible, and the distinction must be maintained throughout.

**The mode of the presenting problem.** A young person who presents in an angry child mode is often, in the first instance, in a system in which the anger is the only available expression. The schema therapist's task with a young person is to get to the vulnerable mode, which is where the unmet need is, and this requires that the young person be able to trust that the vulnerability will be survivable.

**The parent's role.** The parent is usually a significant part of the schema's maintenance, and the theory's stance is that the parent's own schema and modes are part of the picture. The work with the parent is a sub-field in its own right, and the ethical care described in Section 10 applies.

**The specificity of the developmental adaptations.** The research on the childhood and adolescent applications of schema therapy is considerably thinner than the adult research, and the adaptations have been made in practice more often than they have been evaluated. A student should treat the developmental adaptations as clinical judgement informed by the theory, not as an evidence-based protocol.

## 8. Evidence and Research Support

**The evidence base is substantial and growing, and it is stronger for the adult treatments than for the developmental applications.**

**Personality disorders.** The schema-focused treatments for borderline personality disorder and for avoidant personality disorder have been evaluated in a series of randomised controlled trials, with substantial effects on both the symptoms and the schema-related constructs, and the evidence is among the more convincing in the personality disorder literature. The larger and better-powered trials have been conducted in adults.

**The construct validity of the schemas.** The question at the centre of the Beck–Young dispute has been answered substantially in favour of Young. The self-report and clinician-rated measures of the schema constructs have demonstrated good reliability and convergent, discriminant, and construct validity across a substantial international literature. The schemas have been shown to be predictive of treatment outcome, to be associated with a range of psychopathology, to be stable over time, and to be culturally modulated.

**The outcomes.** Meta-analyses of schema therapy across conditions report moderate to large effects, and the therapy has been evaluated in depression, anxiety, personality disorder, eating disorder, chronic pain, and other conditions. The comparison with standard cognitive therapy in several trials has shown differences in some measures and not in others, and the honest summary is that schema therapy adds value in the characterological presentations and is broadly comparable in others.

**The developmental applications.** The evidence for schema therapy with children and adolescents specifically is thin. A small number of studies, mostly uncontrolled or with small controlled samples, have been conducted with adolescents with eating disorders, with self-harm, and with personality disorder features, and the results are promising enough to justify further work. The developmental adaptations, including the specific work with the child version and the modifications to the chair work, are described in the clinical literature and have not been systematically evaluated.

**The limits.** The allegiance problem persists, and a substantial proportion of the studies have been conducted by investigators with an allegiance to the theory. The constructs are numerous, and the theoretical breadth of the model creates a risk of unfalsifiability that Beck's original objection identified, and which the validation research has substantially but not completely addressed.

## 9. Strengths and Limitations of Schema Therapy

**Strengths.** It addresses the characterological level, which the standard cognitive therapies do not reach, and it does so with a model that is unusually precise about the developmental origins of a young person's difficulties. The mode model gives a young person an immediately usable description of their own experience, and the maps are often the turning point in the work. The techniques — imagery rescripting, empathic confrontation, chair work, dialogue work — are well specified and are among the most usable in the field. The theory is compatible with the developmental literature, with the attachment research, and with the emotion-focused and somatic traditions, and the integration is productive rather than merely fashionable. And the emphasis on the unmet need behind the difficulty is a humane and clinically useful orientation that is often absent from the other characterological approaches.

**Limitations.** The developmental evidence is thin, and the adult evidence is not transferable without modification. The theory's constructs are numerous and the risk of unfalsifiability is real, and a student who uses the theory loosely will produce an unfalsifiable and unhelpfully general account of a young person. The reparenting specification is ethically delicate and requires careful handling, and a practitioner who is unclear about the limits will do harm. And the model's breadth, which is its intellectual strength, is also its practical weakness, because a young person with a simple and clear difficulty can be over-formulated.

# 10. Cultural and Ethical Considerations

## 10.1 Culture and the schemas

**The universality of the schemas.** The schemas are presented as universal, and the research has found substantial cross-cultural consistency in the factor structure of the schema measures, which is a genuine finding. The consistency is nevertheless at the level of the schema categories, and it does not mean that the *content* of a young person's schemas is the same across cultures. A schema of Unrelenting Standards in a culture with a strong achievement ethic may be considerably less maladaptive than in a culture where the same standard is unusual, and the developmental antecedents of a schema of Emotional Deprivation will look different in a family that depends on extended kin for childcare than in a nuclear family.

**The cultural specification of the reparenting.** The theory's specification of the therapist's role assumes a particular form of emotional availability and a particular form of limit-setting. In a cultural context in which the expression of need is understood to be a sign of immaturity, or in which the authority of the professional is not questioned, or in which the family is the primary unit of emotional life, the reparenting may be experienced as intrusion rather than as provision, and the developmental work with the parents is correspondingly more important.

**The risk of pathologising the family.** A young person who has been brought up in a family with high standards and strong expectations may be diagnosed with the Unrelenting Standards and the Subjugation schemas, and the diagnosis may be accurate in a technical sense while being profoundly misapplied, because the standards may be the young person's primary source of belonging. The ethical requirement is that the therapist ask what the schema is *for* before treating it, which is the same requirement that governs the work with the person-centred and the systemic traditions.

## 10.2 The ethical requirement of the bounded reparenting

The reparenting is the element of the theory that most requires ethical specification, and the specification is the following.

**The therapist provides warmth, empathy, validation, and unconditional positive regard.** This is a therapeutic stance, and it is a legitimate one.

**The therapist sets clear, consistent, kindly limits.** The limits are the boundary of the relationship, and they include the practical ones — the time, the frequency, the setting, the availability — and the limits on the young person's use of the relationship.

**The therapist does not become a parent substitute.** The relationship is a therapeutic relationship with a specific function, and the young person's developmental task is to become an autonomous adult, not to acquire a second parent. A practitioner who is unclear about this will find a young person who becomes dependent, and who has been harmed by a therapy that was supposed to help.

**The developmental task of the relationship.** The theory's account of the developmental needs behind the schemas implies that the therapeutic relationship should itself be developmentally appropriate, and that means that as the young person develops, the therapist's role should change, and that the eventual ending of the relationship is a developmental event and not merely an administrative one. Chapter 54 develops this.

**The value of the approach.** The ethical value of the theory is also considerable, and it is worth stating. It is one of the few approaches in the field that is explicitly concerned with the young person's developmental

history, that takes the early relationship seriously, and that gives a young person a way of understanding the difficulties that they have carried since childhood — which is, for many young people, the most relieving thing that has ever happened to them.

## 11. Case Illustration: The Girl Who Was Two Different People

**The referral.** Ines, fifteen, is referred by a child and adolescent mental health service for "self-harm, school refusal, and possible borderline." The referral notes that Ines has cut for eighteen months, that she has been in three schools in two years, and that she "swings between being delightful and being abusive." Her parents are separated; her mother has a new partner; Ines sees her father every other weekend.

**The assessment.** The schema assessment establishes the following.

**The primary schema: Abandonment/Instability.** The developmental antecedents are the father's departures, the moves, and the fact that each departure has been followed by a return and by another departure, with the return never quite reliable. The unmet need is for a secure base and for stability.

**The second schema: Defectiveness/Shame.** The antecedents include the mother's emotional reliance on Ines, which began when Ines was nine and her father left, and a family in which Ines's emotional responsiveness was needed by adults. The unmet need is for unconditional acceptance and for permission to have needs of her own.

**The third schema: Emotional Deprivation.** The antecedents are the emotional structure of the household. The unmet need is for emotional warmth and for a sense of being wanted for oneself.

**The coping style: overcompensation, with a strong avoidance component.** Ines is hyper-competent and hyper-control with some adults — the "delightful" mode — and avoidant of others, particularly of the school. Her self-harm occurs in the context of the abandonment schema being activated.

**The mode map.** The "delightful" Ines is a demanding parent mode — the internalised "you must be perfect and you must not need anyone" voice, which is also her mother's voice. The "abusive" Ines is an angry child mode. The vulnerable, sad, desperate Ines is the vulnerable child mode and is the one the young person almost never shows. The healthy adult mode is small and undeveloped, and the practitioner describes it as "a nine-year-old who is very sensible."

**The therapeutic work.** The work proceeds in a specific order, and the order is the clinical skill in this theory.

**First, the map.** The map is drawn with Ines, over two sessions, and the drawing is the intervention. Ines says, looking at it: "That's what I do. I go nice or I go mad and there's nothing in between." The practitioner says: "What's in between?" And Ines finds, with some difficulty, the vulnerable child mode on the map, and says: "That one doesn't get to come out."

**Second, the empathic confrontation.** The confrontation is between Ines's own two positions, and it is done gently. "You've said you want to be completely independent of everybody, and you've also said that if you actually did that, you'd disappear. Both of those are true at the same time, and I want to show you something — which part of you is the one that says 'I must be perfect'?" The answer is revealing: "That's my mum's voice. It's her in my head." And then: "And who is it that is trying to be so good that she never has to need anybody?" And Ines, after a long silence: "Me. At nine."

**Third, the imagery rescripting.** Ines is asked to imagine the scene at nine: her mother crying in the kitchen, Ines coming in to see what was wrong. The scene is familiar and the affect is immediate. The rescripting asks: "What did you need from her that night? What would you have liked her to have said?" Ines: "I wanted her to say 'go to bed, I'll be all right.'" The practitioner asks her to imagine her nine-year-old self hearing that. It takes several sessions, and the practitioner is careful about the pace and about the fact that the nine-year-old needs the experience of a caregiver who is available, and that the imagery is providing it.

**Fourth, the work with the mother.** The mother's own schema and modes are the core of the maintenance, and the mother's "you must be strong" is the demanding parent mode that Ines has internalised. The work with the mother is the hardest part of the case, and the practitioner is explicit that the goal is not to make the mother a different person but to give her something other than her daughter to rely on. The practitioner also raises the question of the father's contact, and Ines's own ambivalence about it, which is complicated by the abandonment schema itself.

**The outcome.** Over nine months, the self-harm reduces to two episodes and then stops; Ines returns to school and stays; and the "delightful" mode becomes less rigid, so that she is occasionally awkward and occasionally disappointing and does not experience this as a catastrophe. The vulnerable child mode is more accessible. The practitioner records that the abandonment schema is not resolved and is not expected to be, and that the work is now about the young person's capacity to tolerate it.

**The teaching point.** The central intervention was not a technique but a map, and the map was made with the young person rather than about her. The order — the map, the confrontation, the rescripting, the parental work — was determined by the theory's account of what maintains the schema, and every element of the order was a decision to work with the protection rather than against it. The case also illustrates the theory's characteristic ethical delicacy, because the mother was doing the very thing that had been done to Ines, and the practitioner had to work with the mother without endorsing what she was doing.

## 12. Key Terms

### Key Terms at a Glance

- **Schema therapy** — A therapy developed by Jeffrey Young that addresses early maladaptive schemas and the modes through which they are enacted, and that is especially suited to characterological and developmental difficulties.
- **Early maladaptive schema** — A pattern of experience and affect, concerning the self and others, formed in childhood and enduring across the lifespan, that is a basis for a wide range of later difficulties.
- **Domains** — The five groups into which the eighteen schemas are organised: disconnection and rejection; impaired autonomy and performance; impaired limits; other-directedness; overvigilance and inhibition.
- **Coping styles** — The three characteristic responses to a schema: surrender, avoidance, and overcompensation.
- **Surrender** — The coping style in which the young person allows the schema to be enacted and thereby confirms it.
- **Avoidance** — The coping style in which the young person avoids the situations in which the schema would be activated.
- **Overcompensation** — The coping style in which the young person does the opposite of the schema in an exaggerated and rigid way.
- **Schema coping cycle** — The sequence in which the schema is activated, the coping style produces short-term benefit, and the long-term consequences confirm the schema.
- **Mode** — A distinct psychological state with its own feelings, thoughts, and behaviours, organised around a particular need or aim.
- **Vulnerable child mode** — The mode in which the young person feels small, sad, scared, ashamed, or helpless, and in which their unmet needs are felt.
- **Angry child mode** — The mode in which the young person feels anger, resentment, or defiance in response to a perceived violation of their needs; a common mode of presentation.
- **Punitive parent mode** — The internalised critical, blaming, and demanding authority; the source of the young person's harsh self-criticism.
- **Demanding parent mode** — The internalised standards that drive performance and prevent the experience of vulnerability.
- **Healthy adult mode** — The mode from which the therapy is conducted; the target of the developmental work.

**Key Terms at a Glance (continued)**

- **Reparenting** — The bounded provision, within the therapeutic relationship, of the empathy, validation, and unconditional regard that the young person did not receive in childhood.
- **Limited reparenting** — The specification of the therapeutic relationship as the bounded provision of what was missing, within clear and consistent limits.
- **The perfect parent model** — The specification of the therapist's stance as warm and validating but also consistently limiting.
- **Empathic confrontation** — The technique of bringing the young person's own conflicting positions into direct contact in a supportive and non-blaming way.
- **Imagery rescripting** — The technique of imagining the original scene and then imagining a version in which the unmet need is met.
- **Chair work** — The technique of a dialogue between the young person and a part of themselves seated in an empty chair.
- **Dialogue work** — The more structured and cognitively oriented version of chair work, in which the conflicting voices are written out and restated.
- **Schema map** — The formulation specifying the schemas, their coping styles, their antecedents, their unmet needs, their triggers, and their consequences.
- **Mode map** — The formulation specifying the modes, their triggers, their triggers, their internal dialogue, and the conditions under which each is entered.
- **Characterological problems** — The pervasive, developmental, and rigid difficulties that the standard cognitive techniques do not reach, and that motivated the development of schema therapy.

## 13. Chapter Summary

Schema therapy was developed by Jeffrey Young out of the finding, documented in Beck's own research programme, that a substantial proportion of patients with characterological difficulties do not respond to standard cognitive therapy. The theory proposes that these difficulties are maintained by early maladaptive schemas — patterns of experience and affect, concerning the self and others, formed in childhood and enduring across the lifespan — that the young person responds to these schemas with one of three coping styles, and that their experience is organised in modes, of which the Healthy Adult is the one from which the therapy is conducted.

The schemas are organised into five domains, and the eighteen schemas are reproduced in Appendix C with their developmental antecedents and their unmet needs. The coping styles — surrender, avoidance, and overcompensation — maintain the schemas through the schema coping cycle, and the clinical implication is that the work is done with the coping style rather than against the schema. The mode model provides a description of the young person's shifting states that is immediately usable with a young person and with their family.

The techniques — limited reparenting, empathic confrontation, imagery rescripting, chair work, and dialogue work — are well specified, and the reparenting specification includes the limits that protect the

young person and the therapist. The theory is unusually developmentally congruent, because the schemas are developmental by definition and the mode model captures the adolescent's experience of shifting between states.

The evidence is substantial for the adult treatments and thin for the developmental applications, and the construct validation research has substantially answered Beck's original objection. The ethical requirements concern the bounded reparenting, the danger of pathologising a family's values, and the requirement that the therapist ask what a schema is *for* before treating it. The case of Ines shows the sequence in action: the map made with the young person, the empathic confrontation, the imagery rescripting, and the difficult work with the mother whose own demand had been transmitted intact.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. State Beck's research finding that motivated the development of schema therapy.
2. Define an early maladaptive schema and explain the significance of each of the five components of the definition.
3. Describe the five domains and give two examples of schemas from each.
4. Explain the three coping styles and the schema coping cycle, and state the clinical implication of the cycle.
5. Distinguish the vulnerable child mode, the angry child mode, the punitive parent mode, the demanding parent mode, and the healthy adult mode.
6. What is limited reparenting, and what are its four components and its limits?
7. Describe imagery rescripting and state its four cautions.
8. Explain why the schema theory is developmentally congruent, and identify the three specific reasons.
9. What does the schema and mode map do that a cognitive conceptualisation does not?
10. State the ethical requirement about the purpose of a schema before treating it, and explain the risk it addresses.

## 15. Practice Activities

**Practice and Application**

### Activity 19.1 — The Mode Map (2 hours, individual, then pairs)

Choose a young person you know — a client, a placement student, a relative — and map their modes. Identify the vulnerable mode, the angry mode, the demanding or punitive parent mode, the coping modes, and the healthy adult mode. Then write the developmental antecedents of the primary schema and the unmet need. Then, in a second pass, write what the young person would need in order for the vulnerable mode to be more accessible. Bring the map to the seminar. The exercise is difficult and the second pass is the part that teaches.

### Activity 19.2 — The Imagery Rescripting Rehearsal (90 minutes, triads)

One student offers a childhood scene from their own life; one conducts a rescripting; one observes for the moment at which the young person's affect changes and for the point at which the practitioner moves too fast. The observer's task is also to note the difference between a rescripting and a re-enactment, because the two are easily confused and the second is a mistake.

### Activity 19.3 — The Purpose of a Schema (60 minutes, written, then pairs)

Take a schema that you or someone you know holds — Unrelenting Standards, Subjugation, Emotional Deprivation — and write, in 300 words, what the schema is *for* in that person's life. Then write what the person would have to give up in order to change it, and what they would have to have instead. The exercise is the ethical core of the theory and it is a direct preparation for the cultural work in Chapter 48.

## 16. Annotated Reading Pathways

**For the model.** Read Young (1990) for the founding of the schema concept, and Young, Klosko, and Weisbarth (2003) for the coping styles and the modes, which is the text a practitioner will actually use.

**For the evidence.** Read the schema-focused personality disorder trials and the meta-analyses, and then read the validation studies of the schema constructs. The history of the dispute with Beck is best understood from the primary literature, and it is one of the more instructive episodes in the profession's history.

**For the developmental applications.** Read the clinical literature on the childhood and adolescent applications with attention to the adaptations and their developmental rationale, and note how much of it is case-based.

**For the related traditions.** Read the object relations literature, the developmental literature on attachment and internal working models, and the emotion-focused literature. The schema theory sits at the intersection of all three, and a student who knows the three will find the theory considerably more coherent.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Schema Therapy: Modes, Styles, and Early Maladaptive Schemas** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is young's model of personality organisation and the schema-focused treatment of adolescents. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The mechanism to keep in

view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Nia is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Nia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Nia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Nia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Nia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Nia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Schema Therapy: Modes, Styles, and Early Maladaptive Schemas

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Schema Therapy: Modes, Styles, and Early Maladaptive Schemas** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Schema Therapy: Modes, Styles, and Early Maladaptive Schemas** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

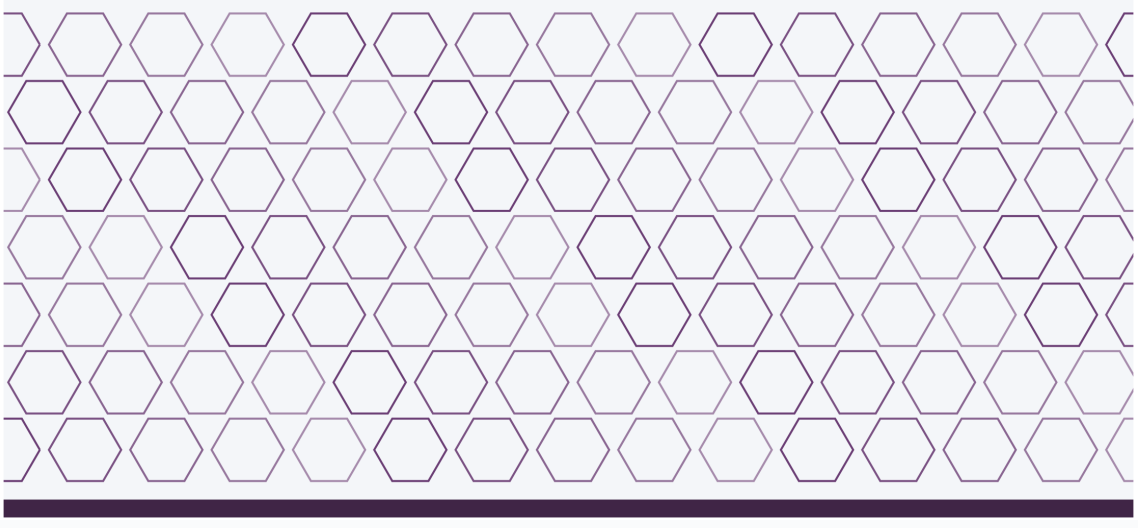
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Schema Therapy: Modes, Styles, and Early Maladaptive Schemas**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Schema Therapy: Modes, Styles, and Early Maladaptive Schemas to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 20

# Acceptance and Commitment Therapy

Psychological flexibility, the hexaflex, and defusion in adolescent work.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the development of acceptance and commitment therapy and explain the context of the debate in which it emerged.
- Explain functional contextualism and relational frame theory at the level required to use the therapy competently.
- Define cognitive fusion, the struggle switch, and the experiential avoidance cycle, and explain the mechanism each describes.
- State the six processes of psychological flexibility and the ACT hexaflex.
- Describe the creative hopelessness perspective and explain its clinical function.
- Describe the principal ACT exercises and state their indications and cautions.
- Apply ACT with children and adolescents, and specify the developmental modifications.
- Evaluate the evidence for ACT and for its components.

## Chapter Overview

Acceptance and commitment therapy was developed by Steven Hayes and his colleagues in the 1980s, and it emerged from a specific and still-running argument within the behaviour therapy movement. The argument was the "acceptance versus change" debate: is a therapeutic success the reduction of symptoms, or is it the construction of a valued life? The behaviour therapy movement had historically been on the side of symptom reduction, and Hayes argued that this commitment had produced a quiet and damaging orthodoxy — the assumption that unwanted internal experiences are medical symptoms to be eliminated, and that a client who has a symptom has a disorder that must be treated before anything else can happen.

Hayes' position, which was and remains unfashionable in parts of the field, is that the struggle to control unwanted thoughts and feelings is itself the mechanism that maintains the problem, and that the answer is not to win the struggle but to stop fighting. The therapeutic task is to develop **psychological flexibility** — the capacity to remain in contact with the present moment and with the person one is, and to act in accordance with one's values, while unwanted internal experiences are present.

For a graduate student in a course on counseling young people, the theory is of interest for three reasons. It is the most fully developed of the third-wave approaches, and it is therefore the theory most likely to be used in a contemporary service. It contains an unusually well-specified account of the relationship between the practitioner's conduct and the young person's development, in the "five senses of the therapist's behaviour." And it offers a specific answer to the question that most often defeats a new therapist working with an adolescent — what do I do when the young person will not do the thing that will help them?

## 1. Historical Context

### 1.1 The acceptance–change debate

The debate within behaviour therapy had three positions. The **traditional position** was that symptom reduction was the goal, and that symptom reduction was necessary before anything more important could be

addressed. The **exposure-based position**, developed in the 1980s by the clinical researchers around the Centre for the Treatment of Fear at the National University of Ireland in Galway, was that a client must approach the feared internal and external stimuli and learn to tolerate them, and that the experience of tolerating them was itself therapeutic, and that avoidance was the problem. The **acceptance position**, advanced by Hayes, was that the trying to control thoughts and feelings was itself the problem, and that the attempt to eliminate symptoms was frequently an avoidance in itself.

The 1990s and 2000s saw the acceptance position become dominant, in part because of the empirical findings of the comparative trials — the acceptance-based treatments generally produced outcomes at least as good as the symptom-focused treatments on the symptoms, and better on the measures of well-being and of functional outcomes.

### 1.2 Hayes' own development

Hayes' path to the theory was unusual and is worth noting, because it explains the theory's character. He was trained in behaviour analysis and spent the early part of his career working on the reduction of self-destructive behaviour, including the treatment of chronic pain, and he was a vigorous opponent of the medical model of chronic pain and of the assumption that the treatment of a "symptom" was the treatment of the problem.

His interest in language and cognition began with his work on the believability of thoughts and on the treatment of depression, and it developed into the relational frame theory, which is a formal account of how human beings relate to language and to each other. The therapy followed from the theory, and the theory followed from the clinical observation that people become trapped in their own struggle with their own minds.

### 1.3 The development of the model

The 1980s and 1990s saw the development of the model, the metaphor of the tug-of-war, the six core processes, and the first controlled trials. The 2000s saw the expansion of the evidence base, the development of the "five senses" account of the therapist's behaviour, the integration with the somatic and trauma fields, and the diffusion of ACT's ideas through the field in the form of acceptance-based behaviour change, which is a more general framework than the therapy.

## 2. The Theoretical Foundations

### 2.1 Functional contextualism

The philosophical position of the therapy is **functional contextualism**, and a student should be able to state it because it explains a great deal of the practice.

Functional contextualism holds that behaviour is best understood in terms of the function it serves in a context, and that the appropriate unit of analysis is the whole person-in-context in relation to a specific problem. It is committed to a metaphysics of practical science: it suspends the question of what is ultimately true in favour of the question of what works, and it is agnostic about the internal causes of behaviour, requiring only that they be operationally defined if they are used explanatorily.

The practical consequence is that ACT asks what is the young person doing, and what is the behaviour accomplishing, rather than what is happening inside them. It is a theory of behaviour in the broadest sense, and it is why the theory has been able to incorporate so many of the field's other findings.

### 2.2 Relational frame theory

The formal account of language is the **relational frame theory**, and a student does not need its details but does need its central claim.

The claim is that human beings learn to relate to words, and to the relations between words, and to the functions of those relations, and that this capacity — rather than any particular content — is what makes the human mind distinctive and what makes a great many psychological problems possible. Specifically, the theory proposes that the capacity to derive the meaning of one event from another is the platform on which the entire behavioural repertoire of the species is built, and that the same platform on which a solution is built is also on which a problem is built.

The clinical application is the concept of **cognitive fusion**: a state in which the person is entangled with the content of their thoughts, in which the thought and the person who thinks it are treated as one and the same, and in which the person's behaviour is governed by the believability of the thought rather than by what is actually happening.

### 2.3 The struggle switch

The **struggle switch** is the central metaphor of the theory, and it is the most useful single idea in it.

The metaphor is of a man standing in a pool with a big rubber ball, holding it under water. The ball is his unwanted thoughts and feelings; the man is the person; and the water is the struggle itself. The man exerts enormous effort, and the effort is the direct cause of the fatigue, the muscular tension, the exhaustion, and the depressed mood. The man believes that letting go of the ball will mean drowning. The ball pushes up; the man pushes down; and the whole system is sustained by the effort.

The clinical claim is that the attempt to control unwanted internal experiences does not only fail to remove them; it produces a set of predictable side effects — the exhaustion, the constriction of behavioural repertoires, the reduced engagement with valued activity, and the worsening mood — and that these side effects are frequently what brings the young person to treatment.

The clinical implication is direct and it is the therapeutic pivot of the whole approach: **the goal is not to change the content of the thoughts and feelings, but to change the young person's relationship to them.** The metaphor, whatever its own problems, communicates this with an efficiency that no other theory in this book has matched.

## 3. The Processes of Psychological Flexibility

### 3.1 The six processes

**Psychological flexibility** is the central construct, and it has six processes.

**Acceptance** is the process of making room for unwanted private experiences without struggle. It is not approval, not resignation, and not the assertion that the experience is good. It is the recognition that the experience is present and that the fight with it is unnecessary.

**Defusion** is the process of seeing thoughts as thoughts. The experiential avoidance strategy of "if I can stop thinking about this, I will be fine" is replaced with the capacity to notice that a thought is occurring, to hold it lightly, and to let it be present without acting on it. Defusion is not suppression and it is not distraction; it is a change in the functional relationship between the person and the content.

**Contact with the present moment** is the process of engaging with the current experience with full attention, in the here and now, without a filter of abstraction, evaluation, or avoidance.

**The self as context** is the process of holding a perspective on one's experience — a sense of "I am the person who is having this thought" rather than "I am this thought" — which creates the possibility of perspective-taking and of a stable, flexible self.

**Values** are the process of determining, consciously and continually, what matters and of choosing a direction of movement that is meaningful.

**Committed action** is the process of taking sustained, concrete action in the direction of the chosen values, with the specific purpose of changing the world rather than the person.

## 3.2 The ACT hexaflex

The six processes are arranged in a hexagonal configuration, in which the three "open" processes on the left — acceptance, defusion, and contact with the present moment — are the processes that loosen the grip of cognitive fusion, and the three "closed" processes on the right — the self as context, values, and committed action — are the processes that move the person in the direction of a valued life.

The flex is the movement around the hexaflex, and it represents the person moving back and forth between loosening and moving, which is the normal pattern of psychological life. The pathological pattern is a *stuck* position: stuck in the closed processes without loosening (which produces rigidity, over-control, and the avoidance of the present moment) or stuck in the open processes without movement (which produces passivity, drifting, and the failure to act on what matters).

The stuck positions map neatly onto the two most common presentations in youth services, and a practitioner who knows the hexaflex can diagnose them quickly. The rigid, over-controlled, perfectionistic, or hyper-competent young person is stuck on the closed side, in the demanding parent mode of Chapter 19 or the demanding mode of the schema model. The young person who is passive, apathetic, dissociated, or profoundly withdrawn is stuck on the open side, having "accepted" everything and moved nowhere.

## 3.3 The creative hopelessness perspective

The **creative hopelessness** perspective is the alternative to the medical model, and it is the ACT analogue of the "depression is not a disease but a problem" formulation.

The claim is that a great many young people's difficulties consist of a single, coherent, and unsuccessful strategy: they have tried to control their internal experiences, in a variety of ways, for a long time, and the strategy has not worked and has made things worse. The evidence for this is the young person's own history, and the therapist's task is to present it to them.

The technique is the **matrix**, a structured comparison of the attempts to control on the one hand and the costs of those attempts on the other. The therapist asks the young person to complete the "Have you tried..." column, and then the "What did it cost you?" column, and the young person produces the evidence themselves. The creative hopelessness follows, and it is the moment at which the therapy's central pivot

becomes available: if controlling has never worked, what else is there?

The clinical significance of the creative hopelessness perspective is that it replaces the moral and the medical framing of a young person's difficulty with a strategic one. The young person is not defective and is not ill; they have been pursuing a strategy that has not worked, and there is another strategy available. This is a very different conversation from the one most young people have had, and it is often the first moment of relief in the treatment.

# 4. The Practice of ACT

## 4.1 Defusion exercises

The defusion exercises are the most characteristic of the ACT techniques, and their function is to demonstrate, experientially, that the content of a thought is not the same thing as the person who is thinking it.

**The "Thank you, mind" or "I'm noticing I'm having the thought that..." exercise.** The young person is asked to notice a thought and to prefix it with the words "I am noticing that I am having the thought that..." The exercise seems trivial and is not: the point is the change in the *relationship* to the thought that the phrasing effects, and the young person frequently experiences a small but real shift, and the practitioner should be careful to notice it and to ask about it.

**The "Tug of war with the mind" metaphor.** The young person is asked to imagine being on one side of a tug-of-war with a personification of their mind, and then to notice what would happen if they dropped the rope. The therapeutic point is the difference between the rope being pulled and the person being dragged. In the classic form, the young person is then invited to notice a second rope, attached to their chest, and to pull that one instead.

**The "Taste it" exercise.** The young person is asked to have the thought and to notice its taste, its texture, its temperature — so that the thought, which is ordinarily experienced as a statement of fact, is experienced as an event. The exercise is very effective with young people who are prone to catastrophising or to rumination.

**The "Milk, milk, milk" exercise.** The young person is asked to say the word "milk" repeatedly until it loses its meaning, and then to notice how long the word takes to regain its meaning. The point is to demonstrate that the meaning of a word is a product of a context, and that a thought's authority is a product of a context too.

## 4.2 The experiential avoidance exercises

**The "Quicksand" metaphor.** The young person is asked to imagine that their difficult thoughts and feelings are quicksand, in which struggling makes it worse and stillness makes it worse too, and that the only workable response is to move slowly. The metaphor is used to establish the acceptance work.

**The "Passengers on the bus" metaphor.** The young person is asked to notice that their thoughts and feelings are like passengers on a bus, and that they are the driver, and that the passengers may be screaming but they do not drive. This is one of the most used and most effective of the ACT metaphors with young people, and it is the one that most directly dismantles the struggle switch.

### 4.3 The values and committed action work

The values work is where ACT departs most sharply from the symptom-focused therapies, and it is also where the developmental adaptation is most needed.

The procedure is: identify what matters; identify the values that are already present in the young person's life; identify the difference between the values and what the young person is currently doing; and then identify the smallest concrete action in the direction of the value.

The question the practitioner must hold throughout is the one the theory insists on: **the question is not "how do I feel about this?" but "what do I do because of this?"** The distinction between feelings and actions is the distinction ACT is built on, and it is also the most clinically useful single question in the therapy with a young person who is stuck.

### 4.4 The "five senses" of the therapist's behaviour

The functional contextual approach specifies the practitioner's conduct in a way that is unusually practical, and it is the part of ACT that most directly addresses the new therapist's central difficulty.

**Evocation.** The practitioner evokes a full range of experience in the young person, including the difficult experience, and does so in a way that allows the difficult experience to be present. The practitioner does not avoid the young person's pain, and does not require the young person to avoid it either.

**Freedom.** The practitioner supports the young person's autonomy, and does so explicitly, with a genuine offer of alternatives and a genuine willingness to be guided by the young person's preferences where these are compatible with the ethical requirements.

**Presence.** The practitioner is fully present, with a stance of curious non-judgement, and treats the young person's experience as a mystery to be explored rather than as a problem to be solved.

**Attunement.** The practitioner attunes to the young person's experience and is willing to be affected by it, and does so without the need to be in control.

**Potency.** The practitioner does things that make a difference, and does so in a way that is responsive to what the young person needs in the moment. This is the element that distinguishes ACT from the non-directive therapies, and it is the one that is most often omitted in practice by therapists who have over-corrected from their earlier training.

The five senses are a description of a stance, and they are also the answer to the question of what a beginning therapist does when the young person will not do the thing that will help them. The answer is: the practitioner is present, is affected, evokes, and does something that makes a difference in this moment.

## 5. ACT with Children and Adolescents

### 5.1 The developmental fit

The developmental fit of ACT with young people is good, and the reasons are specific.

**It respects the young person's difficulty without pathologising it.** The creative hopelessness perspective says that the young person's strategy has not worked, which is a description of a competent person's attempts rather than of a defective person.

**It works with the developmentally available "I don't have to" of adolescence.** The adolescent's developmental task involves the renegotiation of dependence, and ACT's core move — the distinction between the person and the content of the mind — is a developmental achievement that a young person can learn.

**It is compatible with the behavioural activation tradition.** The committed action work and the values work are, in the appropriate cases, behavioural activation, and the two are compatible in a way that the third wave's rhetoric has sometimes obscured.

**It is compatible with play and with the body.** ACT has been adapted to work with young children through play, and the "passengers on the bus" and "quicksand" metaphors have obvious child versions. The somatic and trauma fields have absorbed ACT's ideas about the difference between sensation and evaluation, and the compatibility is genuine.

### 5.2 The developmental modifications

**The metaphor must be developmentally appropriate.** The "passengers on the bus" metaphor is excellent for a ten-year-old and requires adaptation for a sixteen-year-old who will find it childish. The developmental rule is that the metaphor should be pitched at the edge of the young person's capacity and should be able to be taught to a peer, and the practitioner should ask the young person what they think of it before using it.

**The values work must be developmentally framed.** A fourteen-year-old's values are not a list of abstract principles; they are about specific people, specific activities, and specific futures. The work is therefore concrete: "What matters to you?" becomes "Who do you want to still be able to talk to in a year's time?" and "What would you still be doing if nothing changed in your head?" The last of these is the most useful question in the whole therapy.

**The "smaller, better" approach.** ACT with young people requires the therapist to keep the tasks small, the metaphors accessible, and the pace such that the young person can consolidate one skill before the next is introduced. The adult protocol is a twelve-session course and the developmental version is more often a series of brief engagements, and a practitioner who tries to deliver the protocol to a young person will fail.

**The commitment to the present moment.** ACT's contact with the present moment work is very natural with young people, because the young person's own difficulty is often that they are living in an imagined future or an imagined past, and the developmental task of many of the presentations is precisely the development of a usable future orientation.

**The caution on trauma.** ACT is frequently described as incompatible with trauma-focused work, and the concern is real: the acceptance work can be experienced as the requirement to tolerate an unbearable memory, and the defusion work can be experienced as the dismissal of the reality of the abuse. The resolution is the phase-oriented approach of Chapter 38, in which ACT is used in the stabilisation phase rather than in the processing phase, and in which the young person is never asked to defuse a thought about the abuse but is helped to defuse the "I am permanently broken" conclusion that follows from it.

## 6. Modern Developments

**Acceptance-based behaviour change.** The broader framework, which provides the theoretical basis for a wide range of behavioural interventions in health psychology, education, and organisational practice, has

diffused the core ideas throughout the profession. The therapist's language of "how do I feel about this?" becoming "what do I do because of this?" is now widespread.

**ACT in schools and in the community.** A substantial body of work has been done on school-based ACT programmes, and the flexibility of the approach and its compatibility with the brief and preventative models of school work make it particularly well suited to it.

**The integration with the somatic and trauma fields.** The recognition that the acceptance and the exposure approaches are addressing the same thing from different directions — the exposure approach by approaching what is avoided, the acceptance approach by ending the fight with what is present — has led to a productive integration, and the "flexible" exposure work of the trauma field is an obvious descendant of the acceptance work.

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## Further Reading for This Chapter

- Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (1999). *Acceptance and commitment therapy: The process and practice of mindful change*. Guilford. — The primary clinical text.
- Hayes, S. C., Luoma, J. B., & Wilson, K. G. (2009). *ACT: The process and practice of mindful change* (2nd ed.). — The mature version, and the standard reference.
- Hayes, S. C., Barnes-Holmes, A., & Roche, B. (2001). *Relational frame theory*. — The formal theory, for the advanced student.
- Cribb, L., et al. (Eds.). (2018). *Acceptance and commitment therapy: Research and practice*. — The evidence base, comprehensively.
- Kanter, J., et al. (2021). *Acceptance and commitment therapy for adolescents*. — The developmental application.
- Luoma, J. B., & Hayes, S. C. (2021). *The ACT matrix*. — A practical guide to the clinical use of the core methods.

## 7. Case Illustration: The Young Person Who Would Not Do the Homework

**The referral.** Caoimhe, fourteen, is referred by a school for "low mood and refusal." She is not attending school consistently, she has stopped attending her swimming club, where she was previously competitive, and she has been withdrawn. Her mother reports that she "used to be a handful" and has recently become "completely switched off." There is a bereavement in the family six months ago: Caoimhe's grandmother, with whom she had a close relationship, died. Her mother has been off work since the bereavement and is "not handling it."

**The first session.** The practitioner begins, in the ACT tradition, by establishing what Caoimhe has tried. The question is: "What have you tried to make the feeling go away?" And Caoimhe, who is articulate and willing, produces a list with an almost impressive fluency:

- Not thinking about it: "I try to distract myself, watch videos, play games."
- Talking to friends: "I told them I was fine so they wouldn't worry."

- Getting angry: "Sometimes I get really angry and shout at my mum, and then I feel better for about ten minutes, and then I feel worse."
- Cancelling: "I said I was ill, and then I was ill, and then I stopped going and I couldn't go back."
- Trying to be good: "I try to be the same as I was. I do the swimming times and I'm slower and I can't do them, and I don't want anyone to know."

**The matrix.** The practitioner draws the matrix with her — the left column "What I tried" and the right column "What it cost me." She asks Caoimhe to fill in the second column for each of the strategies. And Caoimhe fills it in, and the filling in is the intervention: "Distracting — I don't feel better, I just don't feel it, and then I feel nothing at all, and the nothing is worse." "Telling friends I'm fine — they think I'm fine. I don't call them now." "Getting angry — I'm the person who shouts. My mum doesn't ask me how I am any more." "Cancelling — I don't go back. I don't think I could." "Trying to be good — I'm tired all the time and I hate the pool."

**The creative hopelessness.** The practitioner says: "So none of those has worked. Any of them, for any length of time, actually worked?" And Caoimhe says, after a pause: "No." And then, in a voice that surprises even herself: "Nothing works." And the practitioner says: "That's not you being broken, Caoimhe. That's you having tried really hard at something that doesn't work. Those are different. Hard-working people get stuck when the thing they're working on doesn't work. The problem isn't that you're not trying hard enough. The problem is that trying harder at this is what's keeping you stuck."

**The defusion.** The next exercise is the one that does the work. The practitioner says: "Can I tell you the story about the two people on the bus? There's a man on a bus, and the driver keeps turning around and saying 'you're doing it wrong, you're doing it wrong, you're doing it wrong.' And there's a passenger, and the passenger is screaming 'LEAVE ME ALONE, LEAVE ME ALONE, LEAVE ME ALONE.' And the bus goes nowhere. And the driver is the one who's giving directions, and the passenger is the one who's screaming, and the passenger can't drive. Who do you think is driving your bus?"

Caoimhe says: "Both." And then, after a moment: "No — the screaming. The screaming's driving."

The practitioner does not push the metaphor further. She asks: "What would the passenger look like if it was driving?" And Caoimhe says, slowly: "It'd be the voice that says 'you're not like you were, you're a disappointment, and everyone can tell.'"

**The work with the grandmother.** The practitioner establishes that the voice is the voice of Caoimhe's grandmother, whom she loved, and who is now — in the voice — the one who is disappointed in her. And the practitioner makes the developmental observation explicitly: the grandmother's death has removed the person who would have been able to say "you are not like you used to be, and that is fine." The acceptance work is then reframed: it is not a matter of accepting a truth about her incompetence, because it is not a truth; it is a matter of hearing the grandmother's voice without obeying it.

**The committed action.** The work then turns to the values, and the practitioner uses the concrete version: "You said you were competitive in swimming and that you stopped. If nothing changed in your head — if you woke up tomorrow and your brain was exactly the same — what would you still be doing?" Caoimhe says: "I'd go to the club. I'd go and I'd be slow, and I'd be last, and I'd hate it." "Would you?" "No, I wouldn't." "So what would be true if you went?" "That I'd be there." "That's the question. Not whether you'd be fast. Whether you'd be there."

The committed action is small and specific: to go to the club, to attend two sessions, to swim the distances but not to time them, and to tell her coach in advance. The practitioner and Caoimhe also plan what she will say to her grandmother's voice when it tells her she is a disappointment.

**The work with the mother.** The mother's own withdrawal is the maintaining factor, and the practitioner works with the mother separately, with her consent and with Caoimhe's knowledge. The mother is also in the creative hopelessness: she has been "trying to be strong" and "keeping things normal," and the effect has been to withdraw from Caoimhe entirely. The work with the mother is the same work — the matrix, the defusion, and a small committed action, which is that she will ask Caoimhe one question about the pool and will listen to the answer without trying to fix it.

**The outcome.** Over twelve weeks, Caoimhe returns to the pool and does not swim competitively; her attendance improves but is not consistent; the "voice" is still present but Caoimhe can now name it as the grandmother's, and the naming has changed its relation to her. The mother asks a question and is surprised to find that Caoimhe tells her about the grandmother. Caoimhe says, in the final session of the phase: "I'm not where I was. But I'm not in the same place either. I don't know who I am now. But I know I want to go back."

**The teaching point.** The intervention was not the "return to swimming" goal, which a behavioural activation practitioner might also have set, and it was not the acceptance of the loss, which a grief-focused practitioner might also have offered. It was the demonstration, produced by Caoimhe herself in the matrix, that none of her strategies had ever worked, and the consequent permission to try a different relationship with the content of her mind. The developmental framing of the values question — "if nothing changed in your head" — is what made the work possible for a fourteen-year-old, and it is the ACT analogue of the developmental modification described in Section 5.2.

## 8. Evidence and Research Support

**The evidence base for ACT is among the largest in the profession, and it is the largest of any of the third-wave approaches.**

**The number and quality of the trials.** ACT has been evaluated in a large number of randomised controlled trials across a wide range of conditions and populations, including chronic pain, anxiety, depression, obsessive-compulsive disorder, psychosis, eating disorders, substance use, and a range of health conditions. Several large multisite trials have been conducted, including a major trial in the United Kingdom and the Netherlands in the treatment of psychosis, and the Australian national programme for youth depression, which used ACT-informed treatment.

**The comparison with CBT.** In head-to-head trials, ACT and CBT produce comparable outcomes on the symptoms, and ACT tends to produce somewhat better outcomes on measures of well-being, of psychological flexibility, of work and relationship functioning, and on the measures of values-based action. The differences on the symptom measures are generally small and are not always statistically significant, and the honest summary is that ACT and CBT are broadly comparable treatments with different emphases and with a modestly different profile of benefit.

**The adolescent literature.** The evidence for ACT with adolescents is smaller than the adult evidence but is growing and includes a substantial number of controlled trials in school settings, in the treatment of anxiety, of depression, of disordered eating, and of substance use. The trials are generally well conducted, and the

effect sizes are moderate. The developmental adaptations are described in the clinical literature and the protocol-based evidence is less developed than in the adult case.

**The component evidence.** The studies of the acceptance and commitment components have produced a small body of evidence that the "creative hopelessness perspective" and the "defusion" components contribute to outcome in a way that is separable from the general effect of the therapy. The evidence is not strong, and it is not conclusive, and a student should describe it accurately.

**The limits.** As with the other third-wave approaches, the population problem is real: the samples are disproportionately adult, female, white, and without significant comorbidity, and the research in schools and in the juvenile justice settings is a smaller body with the same methodological qualities. A substantial proportion of the studies have been conducted by investigators with an allegiance to the theory, though the sheer volume of the literature and the number of independent investigators has made this less damaging than it has been elsewhere. The long-term follow-up data are limited.

## 9. Strengths and Limitations of Acceptance and Commitment Therapy

**Strengths.** It provides the clearest and most usable account in the field of why a young person who tries to control their internal experiences makes things worse, and the account is one that a young person can be helped to see for themselves, which makes it extraordinarily well suited to work with the young people who most resist a traditional symptom-focused treatment. The five senses of the therapist's behaviour give a beginning therapist a usable answer to the question of what to actually do, and the theory is unusually coherent as a piece of philosophy, psychology, and practice. The committed action and values work integrate the acceptance and the behavioural traditions rather than opposing them. And the evidence base is large and of good quality.

**Limitations.** The theory's philosophical sophistication is a barrier: the vocabulary of functional contextualism and relational frame theory is a genuine obstacle to training and to multidisciplinary working, and the field has frequently responded by teaching the techniques without the theory, which produces a practice that is often unrecognisable from the theory. The commitment to values work can become coercive if the practitioner has values of their own to impose, and this is a real risk with young people, who are developmentally in the process of determining their own values. The theory's relative neglect of the body, of the developmental context, and of the social and structural determinants is a genuine weakness. And the popular version of the theory, in which acceptance is confused with resignation, is a serious misrepresentation that the profession has not adequately addressed.

## 10. Cultural and Ethical Considerations

### 10.1 Culture and acceptance

**The cultural specification of the struggle.** The theory's account of the struggle switch describes a specific pattern: a person who attempts to control their internal experiences and who is thereby harmed by the attempt. Whether this pattern is universal is an open question. Some features of the account are very widely attested: the tendency to suppress unwanted thoughts, the rebound effect, the narrowness of the behavioural repertoire that suppression produces. Others are more culturally marked.

**The value of control as a cultural good.** In a cultural context in which emotional restraint, in-group conformity, and the performance of composure are strongly valued — and in which the expression of distress is understood to have consequences for the young person and for their family — the struggle with one's own experience may be considerably more adaptive than the theory's account allows. A young person who suppresses their distress in front of a family that would be harmed by the display is not making a cognitive error; they are exercising a difficult and costly skill.

**Values and the developmental task.** The values work is the most culturally sensitive component, because the values in question are assumed to be the young person's own. In a family that organises the young person's life around filial duty, the "values" that the young person reports are not a psychological possession but a relational position, and the values work can become an instrument of the individualisation of a young person whose developmental task is not to separate from the family but to find a way of being a full member of it while retaining their own voice.

**Defusion and the reality of harm.** The defusion work assumes that thoughts are events rather than facts, and the assumption requires qualification where the young person's thinking is a report of something that is actually happening. A young person who thinks "I am going to be hurt" and who is in fact in a dangerous situation must not be helped to defuse that thought into a mere event. The ethical requirement is that the practitioner know the difference, and the clinical safeguard is the comprehensive assessment that every good assessment includes.

## 10.2 The ethical requirements of the values work

**The young person must not be directed towards the practitioner's values.** The temptation is real and the practice is well known. A practitioner who is moved by a young person's suffering may guide them towards a value — self-reliance, expression, self-determination — that is the practitioner's rather than the young person's, and the young person may adopt it, and it may not be theirs.

**The young person must be able to decline.** The committed action must be genuinely chosen, and the young person must be able to say no. And the therapist must be able to work with a young person whose values genuinely conflict with what the young person wants, which is a more frequent situation in adolescence than in adulthood, because the adolescent's values are often genuinely in conflict with each other and with their wishes.

**The "I don't have to" must be a real option.** The core of the therapy's offer is that the young person does not have to fight their thoughts. If the young person is in an environment in which they do in fact have to fight — in which the suppression of distress is required, in which the expression of a need has consequences, in which the safety of the young person depends on their performance — then the therapy's central offer is not available to them, and the practitioner must say so.

**Autonomy and the developmental task.** The theory's commitment to autonomy is in tension with the developmental reality of a young person who is not yet capable of autonomous decision-making in all domains, and the tension is the same one that runs through the whole of this book. The practical resolution is the developmental framing described in Section 5.2: the values work is about the young person's own concrete commitments, not about abstract principles.

## 10.3 The ethical value of the theory

The ethical value of ACT with young people is considerable and is worth recording. It is a therapy that says to a young person: you are not broken, you have been trying something that does not work, and there is a way of relating to your own mind that is different. For a young person who has been told that their anxiety is a illness to be cured, or that their mind is the problem, that is a considerable gift. And the theory's insistence on the practitioner's presence, attunement, and potency is a corrective to the therapeutic minimalism of much of the contemporary humanistic practice.

## 12. Key Terms

**Key Terms at a Glance**

- **Acceptance and commitment therapy (ACT)** — A therapy developed by Steven Hayes in which the aim is psychological flexibility rather than symptom reduction, and in which the struggle to control unwanted internal experiences is identified as a primary maintaining mechanism.
- **Functional contextualism** — The philosophical position in which behaviour is understood in terms of its function in a context, and the practical question is what works.
- **Relational frame theory** — The formal account in which the human capacity to relate to language and to the relations among words is the platform on which both solutions and problems are built.
- **Cognitive fusion** — The condition in which a person is entangled with the content of their thoughts, so that the thought and the person are treated as identical.
- **The struggle switch** — The metaphor in which the effort to suppress unwanted thoughts and feelings is itself the cause of exhaustion, constriction, and reduced engagement.
- **Experiential avoidance** — The attempt to escape from or suppress unwanted internal experiences, and the mechanism that maintains many difficulties.
- **Psychological flexibility** — The capacity to remain in contact with the present moment and with the self-as-context while unwanted internal experiences are present, and to act in accordance with values.
- **The six processes** — Acceptance, defusion, contact with the present moment, the self as context, values, and committed action.
- **The ACT hexaflex** — The hexagonal arrangement of the six processes, with the three "open" processes loosening cognitive fusion and the three "closed" processes moving toward a valued life.
- **Stuck positions** — The pathological patterns in which the person is stuck in the closed processes without loosening, or in the open processes without movement.
- **Creative hopelessness** — The perspective in which the young person's difficulties consist of a coherent and unsuccessful strategy of controlling internal experience, which is demonstrated rather than asserted.
- **The matrix** — The structured comparison of the attempts to control and the costs of those attempts, from which the creative hopelessness follows.
- **Defusion** — The process of seeing thoughts as thoughts; distinct from suppression and from distraction.

**Key Terms at a Glance (continued)**

- **The "passengers on the bus" metaphor** — The ACT metaphor for the distinction between the person and the content of the mind, and probably the most used of the metaphors.
- **The "quicksand" metaphor** — The metaphor for the difference between struggling, stillness, and slow movement in the presence of an unwanted experience.
- **The "milk, milk, milk" exercise** — The demonstration that the meaning of a word is a product of a context.
- **The five senses of the therapist's behaviour** — Evocation, freedom, presence, attunement, and potency; the specification of the practitioner's conduct.
- **Committed action** — Sustained, concrete action in the direction of a chosen value, aimed at changing the world rather than the person.
- **Values-based living** — The orientation in which the person's choices are guided by explicitly determined values rather than by the avoidance of discomfort.
- **The "if nothing changed in your head" question** — The developmental adaptation of the values work, in which the young person is asked what they would still do if their internal experience were unchanged.
- **Flexible exposure** — The trauma-informed descendant of the acceptance work, in which the young person approaches the avoided material while being helped to relate to it differently.

## 13. Chapter Summary

Acceptance and commitment therapy emerged from the acceptance-versus-change debate within behaviour therapy, and its central claim is that the attempt to control unwanted thoughts and feelings is itself the mechanism that maintains many difficulties. The theory rests on functional contextualism, which asks what behaviour is accomplishing rather than what is happening inside the person, and on relational frame theory, which locates the human capacity for problem-construction on the same platform as the capacity for solution-construction.

The clinical core is the struggle switch, the account of a person trying to hold a ball under water and being exhausted by the effort, and the therapeutic pivot is the recognition that the goal is not to change the content of the mind but to change the young person's relationship to it. The six processes of psychological flexibility are arranged in the ACT hexaflex, and the pathological patterns are the stuck positions: the rigid, over-controlled young person stuck on the closed side, and the passive, dissociated young person stuck on the open side.

The creative hopelessness perspective replaces the medical and the moral framing of a young person's difficulty with a strategic one, and the matrix is the technique by which the young person produces the evidence themselves. The practice includes the defusion exercises, the experiential avoidance metaphors, and the values and committed action work, and it is specified for the practitioner by the five senses of the therapist's behaviour.

The developmental fit with young people is good, and the modifications are concrete: pitch the metaphor at the edge of capacity, frame the values work developmentally, keep the tasks small, and use the acceptance work with care in the presence of trauma. The evidence base is among the largest in the profession and is broadly comparable to cognitive therapy on symptoms and somewhat better on well-being and functioning. The ethical requirements concern the young person's right to decline, the practitioner's own values, the assumption that the struggle is always maladaptive, and the requirement to know the difference between a thought and a report of something that is actually happening.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. State the acceptance–change debate and the three positions within it.
2. Explain functional contextualism and state two practical consequences for the practice.
3. Define cognitive fusion and explain how it differs from a false belief.
4. Explain the struggle switch metaphor and the four side effects of the struggle.
5. State the six processes of psychological flexibility and the arrangement of the hexaflex.
6. Describe the two stuck positions and give an adolescent example of each.
7. Explain the creative hopelessness perspective and the function of the matrix.
8. Describe the "passengers on the bus" exercise and the "milk, milk, milk" exercise, and state what each demonstrates.
9. State the five senses of the therapist's behaviour and explain what each requires of the practitioner.
10. State the two ethical requirements of the values work with a young person.

## 15. Practice Activities

**Practice and Application**

### Activity 20.1 — The Matrix (2 hours, individual, then pairs)

Complete the matrix on your own situation: the "what I tried" column, and then the "what it cost me" column, and then the "what actually worked" column. Read the three columns in sequence and notice the creative hopelessness when it arrives. The exercise is the theory's central technique and it works on the practitioner as well as on the client.

### Activity 20.2 — The Metaphor Repertoire (90 minutes, groups of four)

Each group takes one ACT metaphor and produces three versions: for a nine-year-old, for a fourteen-year-old, and for a seventeen-year-old who will find the child version embarrassing. Then they specify the developmental reason for each adaptation. The groups then present the three versions to a peer in the role of a young person, and the peer's feedback on which version they would tolerate is as useful as the group's reasoning.

### Activity 20.3 — The "If Nothing Changed" Question (60 minutes, pairs)

Ask a partner the question: "If nothing changed in your head — if you woke up tomorrow and your brain was exactly the same — what would you still be doing?" Then ask: "Who does that matter to?" and: "What would be the smallest thing you could do this week?" The third question is the ACT one; the second is the developmental one. Then swap. The exercise is a compact version of the therapy and it demonstrates why the values work has to be concrete.

## 16. Annotated Reading Pathways

**For the theory and the practice.** Read Hayes, Luoma, and Wilson (2009) for the whole, and read the ACT Matrix by Luoma and Hayes for the practical use of the core methods. Read the functional contextual account of the therapist's behaviour with attention, because the five senses are the most immediately useful part of the theory for a new practitioner.

**For the evidence.** Read the comprehensive ACT research volume and the head-to-head comparisons with cognitive therapy. Note the pattern of the findings — comparable on symptoms, different on functioning — and consider what it implies for the choice between them.

**For the developmental application.** Read the adolescent ACT literature, and then read the school-based trials, which are the most relevant to a practitioner in a school setting and the most liberally available in the professional literature.

**For the critique.** Read the philosophical critiques of functional contextualism and the cultural critiques of the values work, and read the trauma literature on the misuse of acceptance work with young people who have been abused. The theory's own literature on the phased application of ACT in trauma treatment is the appropriate corrective.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Acceptance and Commitment Therapy** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is psychological flexibility, the hexaflex, and defusion in adolescent work. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how

consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The mechanism to keep in view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Marcus is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Marcus's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Marcus explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Marcus chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Marcus has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Marcus can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Acceptance and Commitment Therapy

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Acceptance and Commitment Therapy** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Acceptance and Commitment Therapy** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

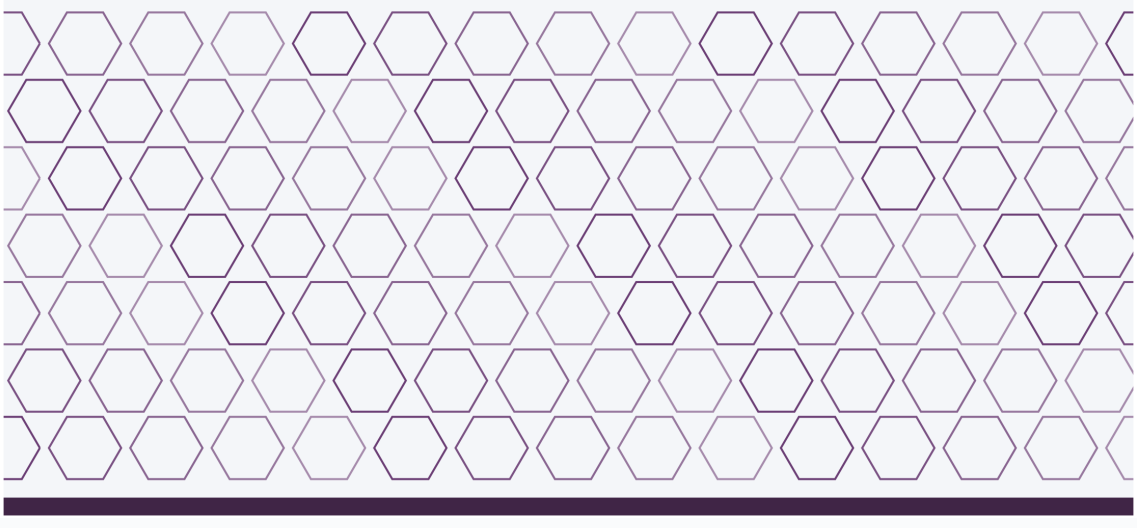
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Acceptance and Commitment Therapy**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Acceptance and Commitment Therapy to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 21

# Dialectical Behavior Therapy and Its Adolescent Adaptations

Linehan's biosocial theory, the four modules, and dialectical strategies in practice.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the origin of dialectical behaviour therapy in the treatment of chronic self-harm and suicidality, and explain the clinical problem it was designed to address.
- Explain the biosocial theory of the development of emotional vulnerability, and state the principal constructs.
- State the core dialectical strategies, the validation strategies, and the problem-solving strategies, and describe how they operate together.
- Describe the four skills modules — mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness — and the four levels of each.
- Describe the structure of a standard dialectical behaviour therapy programme and the consultation team model.
- Describe the adolescent adaptations of dialectical behaviour therapy and their empirical status.
- Apply dialectical behaviour therapy thinking to a case involving a young person.
- Evaluate the evidence for dialectical behaviour therapy and for its adolescent adaptations.

## Chapter Overview

Marsha Linehan developed dialectical behaviour therapy in the late 1980s, in a clinic in Washington State, for a population that the mental health system had largely given up on: women who cut themselves, who had attempted suicide, and who were being discharged from hospital after every admission. Linehan's own description of the difficulty — that the usual therapies, and the usual hospital responses, had made things worse for these patients — is the founding observation of the theory, and it is a critique of the profession that a student should read carefully, because it applies far beyond the population in which it was developed.

The therapy's central technical insight is that the patients who self-harm are in a state of **emotional dysregulation** in which the problem-solving system is offline, and that the clinical task is to move the young person from an emotional, reactive state into a problem-solving state before anything else is attempted. Everything else in the therapy follows from that: the emphasis on the dialectic of acceptance and change, the validation of the young person's experience, the skills training in four modules, the use of a "line" team that speaks with a single voice, and the deliberate assumption that the young person will continue to self-harm and that the work is therefore to make the self-harm less lethal until it stops.

The adolescent adaptations are among the most significant developments in the field, and the DBT for adolescents programme is one of the most widely implemented treatments for adolescent self-harm in the world. Their evidence is good but not as good as the adult evidence, and a student should be able to say so precisely.

## 1. Historical Context

### 1.1 The problem Linehan was solving

Linehan's account of the problem, as she set it out in the early 1990s, is worth stating in her own structure because it identifies a failure that has general application.

Patients who self-harm were typically treated with a combination of supportive therapy, medication, and hospitalisation, and the outcomes were poor: high rates of re-attempt, high rates of discharge followed by re-admission, and a pattern in which the patients were regarded as manipulative and as having brought their difficulties on themselves. The system, in effect, was responding to the behaviour rather than to the distress, and the behaviour was being reinforced by the response.

The alternative treatment, Linehan observed, was not better, because it treated the self-harm as a symptom to be eliminated and ignored the emotional state in which the self-harm occurred. The patients felt that the treatment was a judgement on them, and the therapeutic alliance deteriorated, and the patients left.

The treatment that Linehan built was designed around three observations: that the patients were ambivalent about their own self-harm and wanted to stop but could not; that the self-harm functioned to regulate intolerable emotion; and that a patient who was in an emotional state could not learn, and that the treatment had to address the emotional state first.

### 1.2 The suicide risk assessment and the "nonsuicidal self-injury" distinction

Two developments accompanied the therapy and are clinically important.

The first is the **Suicide Risk Assessment (SRA)**, developed by Linehan and refined with colleagues, which is a clinical assessment of suicide risk that is conducted with the patient rather than about them, that explores the reasons for living and the reasons for dying, and that produces a collaborative safety plan. It has become one of the most widely used and most carefully evaluated risk assessment tools in the world, and Appendix H provides the documentation framework for it.

The second is the **distinction between suicidal self-injury and nonsuicidal self-injury**, which the treatment made explicit and which has become a standard distinction in the field. The distinction matters because the two are associated with different outcomes, the assessment differs, and the young person who engages in nonsuicidal self-injury is frequently not at imminent risk of suicide but is frequently at high risk of later suicidal behaviour, and it is frequently dismissed by adults as "attention-seeking" when it is in fact a regulation strategy that works in the short term.

### 1.3 The development of the programme

The first trial of the standard dialectical behaviour therapy programme was published in 1991 and reported a large effect on the frequency of suicidal behaviours and on the frequency of nonsuicidal self-injury, in a population of women who had been hospitalised specifically for these behaviours. The programme was a year-long, with individual therapy approximately once a week, a skills group once a week, a consultation team meeting once a week, and telephone coaching between sessions. The treatment was the first in the field to be delivered in this structured form, and the breadth of the programme is one of its defining features.

## 2. The Biosocial Theory

### 2.1 The two factors

The theory holds that a young person who develops a chronic pattern of suicidal or self-injurious behaviour has, typically, a **vulnerability** factor and an **environmental** factor, and that the behaviour emerges from their transaction.

**The vulnerability factor** is a biological predisposition, typically involving emotional sensitivity, high reactivity, high intensity, and a slow or inconsistent return to baseline after an emotional activation. Linehan's account is that this is a temperament, that it is substantially inherited, and that it is not a disorder.

**The environmental factor** is a pattern of invalidating environments. The original formulation specified three: a family that is emotionally chaotic and that does not model effective emotional regulation; a family that teaches the young person to inhibit expression of emotion; and a family in which communication of distress is met with escalating responses.

## 2.2 The problem of invalidation

The account of **invalidation** is the heart of the theory and the part that is most often misunderstood.

Invalidation is a set of assumptions about a person that are inconsistent with external reality. The classic examples: "You can't feel that bad, other people have it worse"; "You're being dramatic"; "You don't have to feel that way"; "You're making it up so people will pay attention." Invalidation is not the same as criticism, and it is not the same as a lack of support. It is the attempt to impose on a person a description of their experience that does not fit their experience.

The consequence of a chronic invalidating environment is that the young person does not learn to identify, to label, and to modulate their emotions, because the labelling of their emotional experience has been systematically undermined. The young person becomes **emotionally dysregulated**: intense, slow to return to baseline, and prone to acting from the emotion rather than from a problem-solving state.

## 2.3 The Dialectic

The **dialectic** is the central philosophical and clinical idea, and it is worth stating carefully because it is usually reduced to a slogan about two opposites.

The therapeutic stance is the synthesis of acceptance and change. The therapy accepts the young person's behaviour as it is, without requiring it to change in this moment and without implying that it is acceptable in the long term; and it requires change, in the form of the development of skills that the young person does not currently have and that they need in order to have a life they are willing to live. The therapist does not choose between acceptance and change; the therapist does both at the same time, and holds them together.

Linehan's formulation of the therapeutic relationship in this context is distinctive: the therapist is a **dialectical** stance, and the dialectic is of "both/and" rather than "either/or." The therapist is warm and firm, and the two are not in tension, because the warmth is what makes the firmness possible.

## 2.4 The development of the problem

The theory's account of how the problem develops is worth following, because it is a developmental account and it connects the theory to the material in Chapter 9.

The young person has the vulnerability factor. The environment invalidates. The young person's emotions are therefore not validated and are not developed, and the young person's emotional regulation capacity is impaired. The young person is therefore, in the presence of an emotional trigger, in a state of emotional

dysregulation: intense emotion, no access to problem-solving, and a very high probability of an impulsive, harmful action. The self-injury functions to modulate the intolerable emotion, and it works, in the short term, which is precisely why it persists. The consequences of the self-injury — shame, the need to hide it, the escalation of the family's response, the increased invalidation — produce a further increase in the emotional vulnerability, and the cycle continues.

# 3. The Strategy Set

## 3.1 The three strategy families

The therapy's clinical repertoire is organised into three families, and a student should be able to name them and state the function of each.

**Dialectical strategies** maximise both acceptance and change simultaneously. The most important of them is the dialectical strategy of **validation and invalidation**, which is the direct counter to the invalidating environment that produced the problem. The therapist validates the young person's emotion and their reasoning, and then, if necessary, points out where the reasoning has led to a problem and what the young person might do differently. The classic formulation is: "It makes sense that you want to hit him; it makes sense that you're angry; and it might also make sense to walk away, and here's what that would look like." Both statements are true, and the young person is not asked to abandon the first in order to accept the second.

Other dialectical strategies include the dialectical strategy of **problem-solving first** (which addresses the practical problems before the emotional ones, in order to reduce the emotional pressure) and the dialectical strategy of **devil's advocate** (in which the therapist argues against the young person's position, in order to make the young person argue for it and thereby examine it).

**Validation strategies** make the young person feel understood. The strategies are the accurate reflection of the emotion, the accurate reflection of the reasoning, the communication of understanding of the young person's difficulties as being reasonable given their circumstances, and the engagement in problem-solving as a demonstration of respect for the young person's intelligence.

**Problem-solving strategies** teach the young person to change their behaviour, their thoughts, and their physiological state. They are drawn from the cognitive-behavioural, the behavioural, the exposure, and the mindfulness traditions, and they are applied in the problem-solving state, which is the state in which the young person is not emotionally overwhelmed.

## 3.2 Validation in practice

The validation strategies deserve more attention than they usually receive, because they are the part of the therapy that reverses the developmental damage, and because they are the part that a trainee finds hardest to do well.

**Validating the emotion.** "That makes sense. Of course you're furious. He told everyone, and you trusted him." The statement is about the emotional response, and it is not a statement about the behaviour, and the two must be kept apart.

**Validating the reasoning.** "It makes sense that you think it's not fair. He did promise." The statement is about the logic, and it is given even when the conclusion drawn from the logic is not acceptable, and this is

the move that students find hardest: the reasoning is validated while the behaviour that follows from it is not.

**Validating the difficulty.** "It makes sense that you're finding this hard. You've been doing this for years and it's not working, and people have been telling you it's not good enough."

**Validating the person.** "You're doing something really difficult right now, and you haven't done it before."

**Invalidating.** The use of invalidation in the therapy is deliberate and limited, and a student should understand the rule: the therapist invalidates the *behaviour* and never the emotion, the reasoning, or the person. "Hitting him isn't going to help you here and it's not going to be allowed" invalidates the behaviour. "You're not allowed to feel angry" invalidates the emotion and is a serious error.

# 4. The Skills Training

## 4.1 The four modules

**Mindfulness.** The module on "what" rather than "how" — attending to the present moment with curiosity and non-judgement, and "how" — engaging in the current moment in a way that is more effective than the automatic patterns the young person has developed. The mindfulness skills in the therapy are the specific "observe," "describe," and "participate" skills, and the practice of them is directly relevant to the young person's tendency to act from the emotion.

**Distress tolerance.** The module on surviving a crisis without making it worse. The skills include the TIPP procedures (temperature change, intense exercise, paced breathing, and progressive muscle relaxation), the "STOP" skill, the "pros and cons" analysis, the "improve the moment" skills, and the acceptance and willingness skills. The rationale is that the young person needs a way of getting through the next ten minutes, and that the first step in changing a behaviour is surviving it.

**Emotion regulation** is the module on understanding and managing the emotions themselves: identifying and labelling the emotion, understanding its function and its message, reducing the vulnerability, increasing the positive reinforcement, and opposite action, and the problem-solving and relationship skills that support all of these. The emotion regulation skills are the direct answer to the invalidating environment, because they teach the young person the labelling that was never developed.

**Interpersonal effectiveness** is the module on the other people: being able to ask for what is needed, being able to say no, being able to maintain a sense of self in a relationship, and being able to solve problems in relationships. It is the module most directly responsive to the developmental task of adolescence, and the module in which the developmental adaptations are most needed.

## 4.2 The four levels

Each skill is taught at four levels, and the four-level structure is one of the therapy's most useful teaching devices.

**Level one: the crisis skills.** The skills that are used in the moment of crisis — for example, the TIPP procedures, or the STOP skill.

**Level two: the main skills.** The fuller version of the skill, taught in the group, with practice and homework.

**Level three: the perspective-taking skills.** "What did I do that made this harder?" "How would you have seen this if you were in their position?" "What is the other possible thing that this situation might mean?" The level-three questions develop the young person's capacity for the perspective-taking that the invalidating environment has prevented, and they are directly relevant to the developmental literature on the adolescent's emerging capacity for moral reasoning and for mentalising.

**Level four: the radical acceptance skills.** "This is just the way it is right now. This is the moment I am in. I am doing my best, and so are you, and I am going to be here with you." The level-four skills are the most profound and the most difficult, and they are taught in the context of the young person's own behaviour, in which the goal is to accept reality as it is while changing it.

### 4.3 The homework and the skills sheets

The therapy is heavily reliant on the skills sheets and on the homework, and the homework is the mechanism by which the skills become available in the moment of crisis. The skills sheets are short, and the young person writes on them, and the practice is deliberate and repeated.

## 5. The Programme Structure

### 5.1 The standard programme

The standard programme is a year-long, with four components: individual therapy approximately once a week; a skills group once a week, in which the four modules are taught in sequence; a **consultation team** meeting approximately once a week, comprising the individual therapist, the skills trainer, and the psychiatrist, with others as needed; and telephone coaching between sessions, with the skills trainer available.

The consultation team is one of the therapy's most distinctive features and is a direct answer to the "splitting" problem: patients who self-harm typically divide the staff team, and the consultation team ensures that the team speaks with one voice, and that no member of the team is drawn into a covert alliance with the patient against another member.

### 5.2 The suicide risk assessment and the safety plan

The formal suicide risk assessment is conducted with the patient, it is repeated whenever the risk changes, and it is documented. The **safety plan**, which replaces the traditional no-harm contract in this therapy, is a collaboratively constructed document that specifies the warning signs, the internal coping strategies, the people and places that provide distraction, the people who can help with a problem, the professionals or services to be contacted, and the making of the environment safe. Appendix H provides the templates and the documentation requirements.

## 6. Dialectical Behaviour Therapy with Adolescents

### 6.1 The adaptations

The adolescent adaptations of the therapy were developed by Linehan and colleagues, principally for the treatment of adolescent self-harm and of the range of difficulties that accompany it — depression, anxiety,

substance use, disordered eating, and the family and school difficulties. The randomised trial of the adolescent-adapted programme was published in 2015 and was a large, well-conducted study, and the programme has since been widely implemented.

The principal adaptations are: the parent involvement, with the parents participating in a skills group of their own and in individual sessions, which is the single most important difference from the adult programme; the developmentally adjusted language and content of the skills; the reduction in the length of the treatment from a year to approximately six months, with a planned continuation phase; the increased emphasis on the interpersonal effectiveness module, which is where the developmental changes are most extensive; the use of the family and school as the primary targets of the interpersonal work; and the developmental emphasis on the "one thing at a time" formulation of the change process.

## 6.2 The parent component

The parent component is what makes the adolescent adaptation a family intervention rather than an individual one, and it is the component with the most important developmental logic.

The parents of a self-harming adolescent are, typically, in a state of acute distress, with a well-developed belief that the self-harm is manipulative, that it is attention-seeking, and that the solution is to remove the attention. Many parents are themselves emotionally dysregulated, and many are themselves survivors of the same pattern in their own families, which is the intergenerational transmission that the biosocial theory predicts.

The parent work therefore involves: the parents learning the same skills; the parents learning the validation strategies, which is the direct counter to the pattern in which the young person's distress was met with escalating responses; the parents working on their own emotional regulation, which is the developmental task of the parent's own "adolescent" phase of life, which the parents are frequently still in; and the parents being helped to distinguish between the self-harm and the underlying distress, which is a distinction that many parents cannot make and which is at the heart of the family pattern.

## 6.3 The developmental rationale

The developmental rationale for the parent component is worth stating, because it is the point at which the theory connects to the developmental literature in Chapter 9.

A self-harming adolescent is almost always in a state of developmental arrest: the developmental tasks of the second decade — the establishment of a peer position, the renegotiation of the parent relationship, the development of an identity, the increase in autonomy — have been arrested by the emotional dysregulation and by the family's response to it. The adolescent who is in a state of constant emotional crisis is not working on any of these tasks. The work of the therapy is to restore the developmental process, and the parent component is essential because the parents are the people who have the most control over the conditions in which the developmental process can resume.

## 6.4 The other adaptations and their evidence

The other adaptations include the use of the "middle path" and the "walking and talking" skills for the young person, the specific adaptation of the interpersonal effectiveness module to address peer relationships, the reduction of the treatment length, and the emphasis on the involvement of the school.

The evidence for the adolescent programme is good but smaller than the adult evidence. The 2015 trial was large and well-conducted and found substantial effects on the frequency of self-harm and on the frequency of suicidal ideation, with some erosion of the effect at follow-up. The implementation literature, which is now substantial, shows that the programme is widely used and that fidelity is variable, and that outcomes in routine services are less favourable than in the trial. The systematic reviews of the adolescent evidence base are generally positive, with the standard caveats.

## Further Reading for This Chapter

- Linehan, M. M. (1993). *Cognitive-behavioral treatment of chronically parasuicidal borderline patients*. Guilford. — The primary text, and the source of the biosocial theory.
- Linehan, M. M. (2014). *DBT skills training manual* (2nd ed.). Guilford. — The skills manual, and the standard reference.
- Linehan, M. M., et al. (2015). *Dialectical behavior therapy for adolescents with suicide attempts and self-harm: A randomized clinical trial and its follow-up*. — The adolescent trial.
- Linehan, M. M., Korslund, K. E., & Harned, M. S. (2014). *DBT for safety: Suicide risk assessment and management*. Guilford. — The risk assessment and safety planning.
- Korslund, K. E., Linehan, M. M., & Harned, M. S. (2019). *Handbook of DBT skills training*.
- Swartz, M. (2014). *The dialectical behavior therapy skills workbook* (2nd ed.). — The client-facing materials, and very usable with young people.

## 7. Case Illustration: The Young Person Who Had Run Out of Ways to Make It Stop

**The referral.** Kellie, sixteen, is referred by a child and adolescent mental health service following an admission to a psychiatric inpatient unit after an overdose. Kellie has had eleven admissions in three years, has cut since the age of twelve, and has a history of a diagnosis of borderline personality disorder. The referral notes: "Kellie self-harms when angry. She says she does it 'to feel something' and 'to get it over with.' Mum says she does it 'for attention.' Kellie has disengaged from two previous services."

**The assessment.** The practitioner conducts the suicide risk assessment with Kellie, and the assessment is thorough: the reasons for living, the reasons for dying, the specific plans, the means, the past behaviour, the protective factors, and the warning signs. The risk is assessed as significant but not imminent, and a collaborative safety plan is constructed with Kellie, and it is the first time that Kellie has been involved in constructing one.

**The biosocial formulation.** The formulation is developed in the first three sessions and is shared with Kellie in plain language, because the theory's account is in language that a young person can use.

The vulnerability factor is clear: Kellie is highly emotionally reactive, feels things very strongly, and takes a long time to come down from an emotional peak. She says: "I feel everything at a hundred and everyone else is at about thirty."

The environmental factor is equally clear. Kellie's mother is a single parent of two, working two jobs, who describes Kellie as "attention-seeking" and "manipulative" and who has been told by professionals — repeatedly and over years — that Kellie's self-harm is manipulative. The pattern of escalation is visible in the record: a small upset, a demand for comfort, the mother's exhaustion, a remark about the attention-seeking, Kellie's escalation, an incident, a hospital admission, and a period of calm in the hospital followed by a discharge and a repetition.

**The reframe.** The practitioner does the most important single intervention in the case in the second session. She says: "Can I tell you what I think is going on? I think your mum has been told for years that you do it for attention, and I think she has believed it, because it is what everyone has told her and because it is the only explanation that makes sense. And I think you know that she believes it, because you have tried to tell her otherwise for years. And I think that is why the last thing you said to me was that you are tired of people saying it's for attention, and that I am probably going to say it too."

Kellie is crying. "Yes. Yes. That's exactly it."

"And I want to say something else. When you cut, does it change anything? Does the feeling change?" Kellie thinks. "It... it goes quiet. For a bit." "So it works." "It works for about an hour." "And then?" "And then I'm more of a mess and I've got to hide it and my mum's crying and it's all worse."

"So the thing that's making everything worse is not the cutting. The cutting is the thing that's stopping you from feeling worse for an hour. And then everything is worse. And everybody has decided that the cutting is the problem and the person is the problem, and so you get more of the same."

**The dialectic.** The practitioner then does the acceptance. "And I'm not going to tell you to stop, because I don't think you can, and I'd be insulting you. I'm going to work on the hour. I want the hour to be longer and better, and then the cutting will stop being necessary, and then we can talk about what happens instead."

**The skills.** The work proceeds on the four modules, and the two most important for Kellie are the distress tolerance and the emotion regulation.

The distress tolerance work is concrete: the TIPP procedures, practised until they are automatic, and the "pros and cons" analysis of the impulse to self-harm, done in the session and then as homework, in which Kellie writes the short-term and the long-term consequences of cutting. The short-term consequence is written first and is not minimised: "I feel quiet." The long-term: "My mum cries. I have to wear long sleeves. I have to lie. The next hour is worse." Kellie reads her own homework and says: "I've never written that down before."

The emotion regulation work is the module that addresses the invalidation directly, and the practitioner teaches Kellie the labelling of emotion with a full list and with a visual scale. "I am at 90 out of 100 on anger" becomes a sentence Kellie can say in the house. The practitioner also teaches the opposite action skill, and the validation skills, and the problem-solving skills.

**The parent component.** Kellie's mother is invited to the sessions, and this is the most delicate part of the work. The mother's first reaction to the account of the invalidation is defensive: "I've been told for three years that this is what she does." The practitioner does not dispute this and does not attribute it to the mother; she says: "You were told it, and you believed it, because it was what you were told, and because Kellie was in hospital and you were frightened and you needed an explanation. That wasn't a failure. But Kellie has been living with the belief that she does it for attention, and she is a sixteen-year-old, and she has carried that on her own for three years."

The mother cries in the session, and the practitioner then teaches her the distinction that is at the heart of the family pattern: "When Kellie cuts, what does she get? Not what do you think she gets — what does she get?" And the mother says, slowly: "She gets quiet." "And what do you get when she cuts?" "I get frightened. I get angry. I get alone with it." "So it's the same thing. She's trying to make the feeling stop and you're trying to make it stop. You are doing the same thing at the same time and you are on opposite sides of it."

**The work with the school.** Kellie has been excluded twice. The practitioner contacts the school, with Kellie's consent, and the school is willing to have a "return to school plan" with a named adult, a reduced timetable, and a specific arrangement for what happens if Kellie becomes distressed on site. The practitioner does not conceal the self-harm from the school; she discusses with Kellie what will be shared, and Kellie agrees.

**The outcome.** Over six months, the frequency of self-injury falls by roughly two thirds, though it does not stop. Kellie misses the first few weeks of school and then returns with a reduced timetable, and she stays. The mother attends her own skills group and reports that the biggest change is that she has stopped saying "you're doing it for attention" — and the practitioner says to her, "say that again to me next week, because that sentence is the thing we are working on."

**The teaching point.** The central intervention was a reframe that the young person produced herself, and the central structural intervention was the parent component, which the adult programme would not have had. The acceptance of the continued self-harm — "I'm not going to tell you to stop" — is what made the work possible, and it is the direct answer to the invalidating environment that produced the problem. A practitioner who had started with a safety contract, a prohibition on self-harm, and a referral for family therapy would have reproduced exactly the pattern that had brought Kellie to the service for the fourth time.

## 8. Evidence and Research Support

**The adult evidence is among the strongest in the profession.** The initial 1991 trial was followed by a series of controlled trials, most of them in women with borderline personality disorder with recurrent suicidal behaviour, with substantial effects on the frequency of suicidal behaviour and nonsuicidal self-injury, on the frequency of psychiatric hospital admissions, on the retention in treatment, and on the measures of emotional dysregulation. The most rigorous of these trials included a comparison with a treatment-as-usual control, and the effects on the frequency of self-harm were large. A Cochrane-style systematic review of the treatment of recurrent self-harm concluded that dialectical behaviour therapy was the treatment with the strongest evidence.

**The adolescent evidence is good and growing.** The 2015 trial of the adolescent-adapted programme found substantial effects on the frequency of self-harm and on suicidal ideation, with the effect partially eroding at follow-up, which is a common finding in this literature and which has motivated the development of booster and continuation protocols. The subsequent implementation literature is substantial and shows the programme being widely adopted, with variable fidelity and with routine-service outcomes that are less favourable than the trial outcomes.

**The components.** The component evidence is more limited than for the cognitive therapies, and the individual components of the therapy have not been as systematically dismantled. The trials comparing dialectical behaviour therapy with other structured treatments for borderline personality disorder, including with schema therapy and with transdiagnostic cognitive behaviour therapy, have produced comparable

outcomes with some differences in profile.

**The limits.** The population problem persists, and the trials have been conducted predominantly with young women, which is a limitation for a treatment in a service whose caseload is more diverse. The implementation literature shows that the programme requires a great deal of training and a functioning consultation team, and that fidelity is a significant predictor of outcome. And the long-term trajectory of adolescent self-harm, which is known to be highly variable, makes the outcome literature difficult to interpret.

## 9. Strengths and Limitations of Dialectical Behaviour Therapy

**Strengths.** It was developed for a population that the profession had abandoned, and it has changed the profession's practice with them. The biosocial theory is a genuinely developmental account that connects the emotional dysregulation to the invalidating environment and to the family, and the account of the invalidation is one of the sharpest clinical formulations in the field. The dialectical stance is a model of how to be both warm and firm, and it is the most teachable relational stance in this book. The skills are concrete, teachable, and usable in a crisis, and the skills worksheets are among the best clinical materials in the profession. The safety plan has become a standard of care. And the adolescent adaptation, with the parent component, has changed the standard of care for adolescent self-harm.

**Limitations.** The programme is resource-intensive, and its delivery requires training, a functioning team, and a service organisation that can support it. The evidence base is narrower than the enthusiasm suggests, and much of the adolescent evidence comes from a single research group. The skills training is manualised to a degree that makes individualisation difficult, and a young person who is resistant to the worksheets can be lost. The biosocial theory's emphasis on the invalidating family, while usually accurate, can under-attend to the structural and political conditions that produce the young person's difficulties, and can produce a family-blaming formulation in some cases. And the therapy's philosophical sophistication — the dialectic — is the hardest part to transmit, and a practitioner who has learned the skills without the philosophy will deliver a skills programme with an acceptance frame, which is not the same thing.

## 10. Cultural and Ethical Considerations

### 10.1 Culture and the invalidation account

The account of the invalidating environment assumes that the young person's emotional experience is being denied by people who are close to them, and the assumption is usually correct. But the cultural specification of what counts as invalidation is significant, and a practitioner who uses the concept too readily will misdescribe a family.

Many family practices that appear invalidating in the light of the Western individualist account are in fact culturally coherent. Emotional restraint in the presence of elders, the expectation that a young person will not discuss certain subjects with a parent, the practice of a parent setting aside their own distress in order to maintain the family's stability, and the discouragement of complaint as a form of respect — these are not invalidation in the sense the theory intends, and a therapist who tells a parent that they are invalidating their child's emotion may be describing a mismatch between the two families' cultural frameworks rather than a

dysfunction.

A culturally responsive practice therefore asks the question differently: not "is this family invalidating?" but "what is this family doing, and what does it mean in their framework, and what is the mismatch that is causing the young person's distress?" The theory's own framework accommodates this, because the point of the account is the young person's experience of not being able to access their own emotions, and the cultural question is about the route by which that happens.

### 10.2 The ethical requirements

**The requirement that the assessment precede the intervention.** The suicide risk assessment, the collaborative safety plan, and the documentation of both are ethical requirements and are not optional. Appendix H provides the templates.

**The requirement not to require the young person to stop.** The therapy's acceptance of continued self-harm is a clinical position and it is also an ethical one: requiring a young person to promise not to harm themselves sets up a situation in which the young person has made a promise they cannot keep, and in which the breaking of the promise is a further invalidation. The safety plan is the alternative, and the safety plan is a document about what the young person will do, not about what they will refrain from doing.

**The requirement to include the parents without excluding the young person.** The parent component is developmentally indicated and clinically effective, and it also removes the young person's privacy. The ethical requirement is that the young person is told what will be shared, that the parent work is framed in a way that does not require the young person to disclose, and that the practitioner is alert to the family in which the parent's involvement is itself unsafe.

**The requirement about the confidentiality of the safety plan.** The safety plan names people, and the practitioner must be clear with the young person about what is shared with whom, including with the parents and with the school, and must document the boundaries.

**The ethical value of the theory.** The ethical value of the approach with young people is considerable. It is one of the few therapies in this book that says to a young person: "You are not doing this to manipulate anyone. You are doing it because it is the only thing you have found that works, and I am going to help you find something better, and I am not going to make you promise to stop before we have done that." For a young person who has been told for years that they are manipulative, that sentence is the beginning of the treatment.

## 11. Key Terms

**Key Terms at a Glance**

- **Dialectical behaviour therapy (DBT)** — A therapy developed by Marsha Linehan for the treatment of chronic self-harm and suicidality, organised around a dialectic of acceptance and change and delivered through individual therapy, skills training, and a consultation team.
- **Biosocial theory** — The account in which a vulnerability factor, comprising emotional sensitivity, reactivity, intensity, and slow return to baseline, interacts with an invalidating environment to produce a pattern of self-harm.
- **Vulnerability factor** — The biological predisposition to emotional dysregulation: sensitivity, reactivity, intensity, and slow return to baseline.
- **Invalidation** — A set of assumptions about a person inconsistent with external reality, in which the attempt to convey one's experience is met with a denial of its validity.
- **Invalidating environment** — A family or social environment in which emotional expression is systematically undermined and in which the communication of distress escalates.
- **The dialectic** — The synthesis of acceptance and change, in which the therapist maximises both simultaneously and holds them together.
- **Dialectical strategies** — Strategies designed to maximise acceptance and change at the same time; the most important is the dialectical strategy of validation and invalidation.
- **Validation strategies** — Strategies that make the young person feel understood; the direct counter to the invalidating environment.
- **Problem-solving strategies** — Strategies from the cognitive-behavioural, behavioural, exposure, and mindfulness traditions, applied in the problem-solving state.
- **Problem-solving state** — The state in which the young person can think and act deliberately, as distinct from the emotional state in which behaviour is driven by the emotion.
- **Emotion dysregulation** — The state of intense, slow-to-settle emotion with no access to problem-solving, produced by the invalidation of emotional experience.
- **The four skills modules** — Mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness.
- **The four levels** — Crisis skills, main skills, perspective-taking skills, and radical acceptance skills; the structure within which each skill is taught.
- **Radical acceptance** — The level-four skill of accepting reality as it is, in the context of changing it.
- **Consultation team** — The weekly meeting of the individual therapist, skills trainer, and psychiatrist, which ensures that the team speaks with a single voice and reduces splitting.

**Key Terms at a Glance (continued)**

- **Suicide Risk Assessment (SRA)** — The collaborative risk assessment conducted with the patient, exploring the reasons for living and for dying.
- **Safety plan** — The collaboratively constructed document specifying warning signs, coping strategies, distractions, people, services, and environmental safety; the replacement for a no-harm contract.
- **Nonsuicidal self-injury** — Self-injury in the absence of suicidal intent, which is associated with a distinct profile and a substantial risk of later suicidal behaviour.
- **DBT for adolescents** — The adolescent adaptation, distinguished above all by the parent skills group and the parent sessions.
- **The parent skills group** — The component in which the parents learn the same skills and the validation strategies; the developmental core of the adolescent adaptation.
- **Middle path and walking and talking** — The interpersonal effectiveness skills adapted for the developmental position of an adolescent, in which the negotiation is with peers and parents rather than with a partner.
- **Skills sheet** — The brief written practice sheet that the young person completes, which is the mechanism by which the skills become available in a crisis.
- **Emotion regulation skill of labelling** — The practice of identifying and naming the emotion, which is the skill that the invalidating environment has prevented, and the direct developmental target of the therapy.

## 12. Chapter Summary

Dialectical behaviour therapy was developed by Marsha Linehan for a population that the mental health system had given up on, and its founding observation is a general critique of the profession: that the usual responses to a self-harming patient treat the behaviour rather than the distress, and that a patient who is in an emotional state cannot learn. The biosocial theory accounts for the condition in terms of a vulnerability factor — emotional sensitivity, reactivity, intensity, and slow return to baseline — interacting with an invalidating environment, and the account of invalidation is the clinical heart of the theory.

The dialectical stance is the synthesis of acceptance and change, and it generates three families of strategy: the dialectical strategies, which maximise both simultaneously; the validation strategies, which make the young person feel understood and which are the direct counter to the invalidation; and the problem-solving strategies, which are applied in the problem-solving state.

The four skills modules — mindfulness, distress tolerance, emotion regulation, and interpersonal effectiveness — are each taught at four levels, from the crisis skill through to radical acceptance. The standard programme combines individual therapy, skills training, a consultation team, and telephone coaching, and it replaces the no-harm contract with a collaboratively constructed safety plan.

The adolescent adaptations, whose principal feature is the parent component, are among the most important developments in the field, and their developmental rationale is that the self-harming adolescent's

developmental tasks are arrested and that the restoration of the developmental process requires the parents' participation, since they control the conditions in which it can resume. The evidence is strong for adults, good and growing for adolescents, and the fidelity of delivery is a significant predictor of outcome. The ethical requirements concern the assessment, the documentation, the refusal to require a promise the young person cannot keep, and the careful handling of the parent component's implications for the young person's privacy. The case of Kellie shows the theory's central intervention, a reframe the young person produced herself, and the parent's account of the same behaviour, and it shows what an acceptance-based approach does that a prohibition-based approach cannot.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the clinical problem Linehan was solving and state the three observations the therapy was built on.
2. State the two factors of the biosocial theory and explain how they produce a self-harming pattern.
3. Define invalidation and give three examples, and explain why it is not the same as criticism.
4. Explain the dialectic and state why the therapist does not choose between acceptance and change.
5. State the three families of strategy and describe the dialectical strategy of validation and invalidation.
6. State the four skills modules and the four levels of a skill.
7. What is the consultation team for, and what problem does it address?
8. What is the safety plan, and why is it used instead of a no-harm contract?
9. Describe the parent component of the adolescent adaptation and state its developmental rationale.
10. What are the limits of applying the concept of invalidation across cultures, and what should the practitioner ask instead?

## 14. Practice Activities

**Practice and Application**

### Activity 21.1 — The Pros and Cons (90 minutes, individual, then pairs)

Complete the pros and cons analysis of a behaviour of your own that you would like to change. Write the short-term consequences first, and do not minimise them. Then write the long-term. Then read the two columns in sequence. The exercise is the distress tolerance skill that most changes a young person's relationship with their own behaviour, and it works on the practitioner too.

### Activity 21.2 — The Validation Rehearsal (2 hours, triads)

One student presents an emotion and a piece of reasoning, for example "I am furious and he never listens." One practises the validation: reflecting the emotion accurately, validating the reasoning, validating the difficulty, and validating the person. One observes, specifically for the moment at which the practitioner validates the reasoning and the student interprets it as the practitioner taking the other's side. The observer also notes the moment the student has a genuine shift, which is usually the fourth kind of validation and not the first. The exercise is difficult and it is the most important skill in the chapter.

### Activity 21.3 — The Family Pattern Map (75 minutes, small groups)

Each group takes a family with a young person who self-harms or is in emotional crisis, and maps the escalation pattern in the way the theory does it: the trigger, the young person's attempt to regulate, the adult's response, the escalation, the external intervention, and the period of calm. Then the group identifies the point at which the pattern could be interrupted, and identifies what it would take. The exercise develops the systemic reading that the biosocial theory implies and that a student often has to work out for themselves.

## 15. Annotated Reading Pathways

**For the theory and the practice.** Read Linehan (1993) for the theory, Linehan (2014) for the skills, and the safety planning volume for the risk assessment. The 1993 text is difficult and worth the effort; it is the clearest statement in the field of what a theoretical stance actually does to clinical practice.

**For the adolescent adaptation.** Read the 2015 trial and the subsequent implementation literature, and read the parent skills materials. The parent component is the part of the adolescent adaptation that matters most clinically, and it is the part that is most often delivered incompletely.

**For the skills.** Work through the skills sheets yourself, in order, over a month. The skills are only usable by a practitioner who has practised them, and the worksheets are designed to be used rather than merely read.

**For the ethical questions.** Read the literature on the assessment of suicide risk in adolescents, the ethics of confidentiality with young people, and the child protection literature on self-harm as a safeguarding indicator. The four of these together constitute the ethical core of the work.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Dialectical Behavior Therapy and Its Adolescent Adaptations** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is linehan's biosocial theory, the four modules, and dialectical strategies in practice. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion,

behaviour, skills practice, and relapse prevention. The mechanism to keep in view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Leah is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Leah's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Leah explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Leah chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Leah has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Leah can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Dialectical Behavior Therapy and Its Adolescent Adaptations

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Dialectical Behavior Therapy and Its Adolescent Adaptations** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Dialectical Behavior Therapy and Its Adolescent Adaptations** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

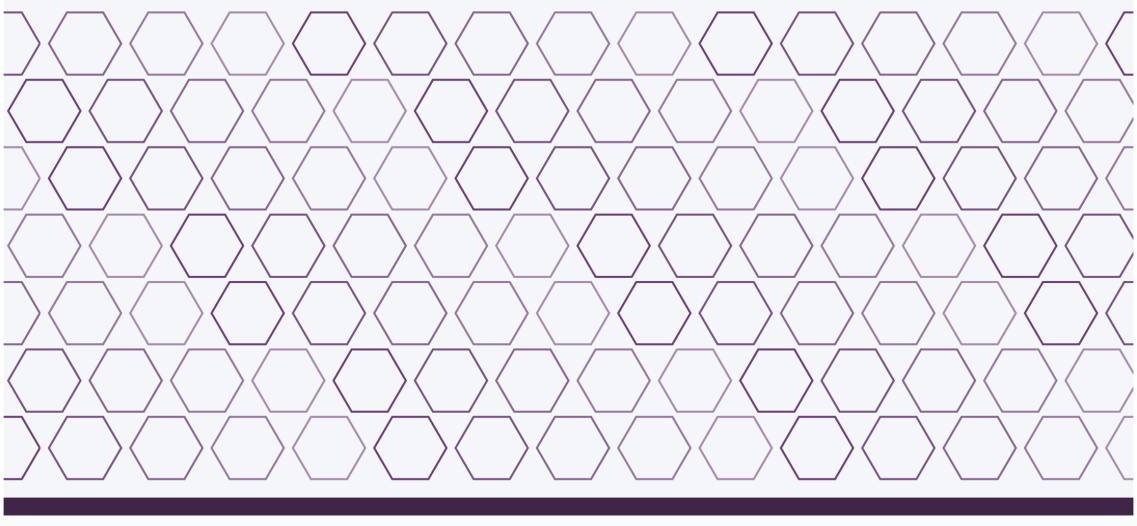
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Dialectical Behavior Therapy and Its Adolescent Adaptations**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Dialectical Behavior Therapy and Its Adolescent Adaptations to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 22

# Mindfulness-Based Cognitive Therapy and Mindfulness Approaches

MBSR, MBCT, and the cultivation of attentional flexibility in young people.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the origin of mindfulness-based stress reduction in the medical clinic and the origin of mindfulness-based cognitive therapy in the prevention of depressive relapse.
- Explain the account of the mechanisms of mindfulness, including the decentering and the reperceiving processes.
- Describe the eight-week mindfulness-based stress reduction course and the eight-week mindfulness-based cognitive therapy course.
- Describe the practices that are common to the mindfulness-based approaches and the practices that are specific to each.
- Apply mindfulness-based work with children and adolescents, and specify the developmental modifications.
- Evaluate the evidence for the mindfulness-based approaches with young people.
- Critically evaluate the mindfulness-based approaches, including their cultural assumptions and their relationship to contemplative traditions.

## Chapter Overview

Mindfulness is the oldest technique in this book and the newest in the profession. It has been practised for some thousands of years in a variety of contemplative traditions, and it entered clinical psychology in the late 1970s in an unexpected place: the stress reduction clinic at the Massachusetts General Hospital, where Jon Kabat-Zinn was asked to develop a programme for patients with chronic pain and stress-related conditions that the medical system could not otherwise help.

The clinical proposition is simple and was for a long time unfashionable: that a person who is systematically trained to attend to the present moment, with a particular quality of attention, will be less at the mercy of the reactivity that maintains distress. The mechanism the tradition proposes is the development of **decentering** — the capacity to observe one's own thoughts and feelings as events occurring in the mind, rather than as facts about the world that require a response.

Mindfulness-based cognitive therapy was developed by Zindel Segal, Mark Williams, and John Teasdale in the 1990s, and it is a specific integration of mindfulness practice with the cognitive model, designed for one purpose: the prevention of depressive relapse. The trial that established it was a well-conducted randomised controlled trial, and it is one of the very few studies in the field that produced a large effect on a clinically important outcome.

This chapter presents the theory, the mechanisms, the courses, the developmental adaptations, the evidence, and — because the question is unavoidable in a graduate course — the cultural questions that the mindfulness literature has engaged with least adequately.

## 1. Historical Context

### 1.1 The medical clinic

Kabat-Zinn's patients had chronic pain, anxiety, and stress-related conditions, and the medical options had been exhausted. He had trained as a molecular biologist and had undertaken extensive study of Zen practice, and his clinical proposition was that the patients' difficulties were maintained by their relationship to their own symptoms — by the struggle, the fear, the catastrophising, and the avoidance that the symptoms generated — and that training in a different relationship to the symptoms would be more useful than any further attempt at symptom removal.

The programme he developed, the mindfulness-based stress reduction course, is described in detail in Section 3. The key feature was that it did not require the patient to be in a state of distress, or to have a particular diagnosis, or to believe in anything. It required eight weeks, two and a half hours a week, and the practice of a defined set of exercises.

## 1.2 The relapse prevention trial

The collaboration between Segal, Williams, and Teasdale at the Oxford Centre for Cognitive Therapy had two features. The first was a programme of mindfulness practice that had been adapted for depressed patients, who have particular difficulty with the practice and who tend to find the present moment painful. The second was the theoretical and clinical argument that the mechanism of depressive relapse is a characteristic pattern of thinking — the reactivation of depressive schemas, and the associated rumination and the reactivation of a general negative thinking style — and that the interval between episodes is the time in which the treatment should be delivered, because it is the time at which the patient's mind is most vulnerable to the reactivation of the old patterns and most able to learn the new ones.

The trial, published in 1990 and replicated several times, compared mindfulness-based cognitive therapy with the continuation of the maintenance antidepressant medication on which the patients were stable, and found that the patients who received mindfulness-based cognitive therapy relapsed at a substantially lower rate over the follow-up period. The subsequent research literature has substantially replicated the finding, with the meta-analyses reporting a substantial effect on relapse rates, and the guidelines in several countries have recommended mindfulness-based cognitive therapy for relapse prevention.

## 1.3 The spread

The 2000s saw the diffusion of the mindfulness-based approaches through the profession, and the diffusion has been remarkable. The methods were applied to stress, to anxiety, to depression, to chronic pain, to cancer, to parenting, to education, to the workplace, to sports, and to the criminal justice system, and the number of published studies grew very rapidly.

The speed of the diffusion has outrun the research, and the field now contains a very large number of small studies of mindfulness-based interventions with heterogeneous protocols, heterogeneous measures, and a substantial risk of publication bias. A student who is asked to evaluate the mindfulness evidence base should be considerably more cautious than the popular accounts suggest.

## 1.4 The contemplative connection

The clinical approaches are not the first application of mindfulness in the West, and a student should know the history. William James's *Varieties of Religious Experience* (1902) and his chapter on the "stream of thought" and on the "lens" of attention, William Brewer's work, and the study of the contemplative practices of Buddhism and of the Christian contemplative tradition all precede the clinical work, and the

contemporary field is now in a complex relationship with these traditions, which is addressed in Section 10.

# 2. The Mechanisms of Mindfulness

## 2.1 The attentional framework

The standard definition of mindfulness is the one developed by the contemporary researchers, and it has two components: attention and orientation.

**Attention** is the deliberate focus on the present moment, with a quality of attention that is non-judgemental. The non-judgemental quality is the operative part, and it is the part that distinguishes mindfulness from ordinary concentration.

**Orientation** is the recognition that the experience of the present moment is itself the object of attention, rather than the object of attention being the content of the present moment. In the ordinary state, the mind is attending to the present moment while the "self" is located in the past and the future — in the recollection of what has happened, the anticipation of what will happen, and the conceptual commentary on both. Mindfulness involves a shift in the orientation of the self toward the present moment.

The theory's claim is that this shift is the mechanism of change, and that it is a change in what the theory calls the **reperceiving** capacity, and that the reperceiving capacity is a trainable skill.

## 2.2 Decentering

**Decentering** is the capacity to see one's thoughts and feelings as events occurring in the mind, rather than as facts about the world, or as accurate summaries of oneself. It is a concept borrowed from the cognitive therapy tradition, where it appears as a technique for holding a thought as a thought, and the mindfulness tradition adopts it as a mechanism.

The two components of decentering are: the ability to perceive a thought or feeling as a mental event rather than as a fact; and the ability to reduce the believability or the impact of that thought or feeling, in the sense of the capacity to step back from it.

The contrast with fusion is the key to the whole theory. In fusion, the thought *is* the reality: "I am anxious" and "anxiety is a fact about my situation" are both experienced as true, and the behaviour is governed accordingly. In decentering, the thought is experienced as a thought: "I am having the thought that I am anxious, and I notice that my chest is tight, and I notice that the thought is about a meeting that is on Thursday."

## 2.3 The eight mechanisms

The contemporary research literature has identified a set of mechanisms that are consistently associated with mindfulness practice, and they are worth knowing because they are what the evidence actually supports, as distinct from what the popular accounts claim.

**Attentional control** — the improved ability to shift and sustain attention, which is the foundation of everything else.

**Attentional awareness** — the ability to notice the contents of experience, which is the mechanism of the early stages of the practice.

**Decentering** — as described.

**Reperceiving** — the capacity to see a situation from a range of perspectives, including the perspective of the wider system and of one's own history.

**Reduction of reactivity** — the reduced tendency to react to stimuli in a conditioned way, which is the behavioural mechanism.

**Response flexibility** — the ability to respond in a way that is appropriate to the situation rather than habitual.

**Mindfulness with acceptance** — the capacity to be present with an unpleasant experience without either suppressing it or reacting to it.

**Values-based action** — the capacity to act in accordance with values, which is the outcome of the practice rather than a mechanism of it.

The components of the mechanisms that matter for work with young people are **decentering** and **reperceiving**, because these are the ones that require a developed metacognitive capacity, and the developmental literature in Chapter 9 is directly relevant to the question of when a young person can learn them.

# 3. The Mindfulness-Based Stress Reduction Course

## 3.1 The eight weeks

The course structure is standard and a student should know it, because the structure is part of the intervention.

**Week one: the body scan.** A 45-minute practice of attending to the body from the feet to the head, and the didactic content is an explanation of mindfulness and of the stress response, and the establishment of the practice of the formal and the informal practice. The body scan is the practice most often used with young people, and it is also the practice that the somatic and trauma-informed fields have adopted.

**Week two: the body scan and the "tuning in" practice.** The didactic content is the exploration of the relationship between thoughts, feelings, and behaviour.

**Week three: mindful movement.** A practice of gentle, deliberate movement — walking, stretching, or yoga — with the attention on the sensations. The didactic content is the development of the observational stance and the demonstration of the difference between reacting and responding.

**Week four: sitting meditation.** The formal sitting practice is introduced, with the attention on the breath and on the arising and passing of thoughts. The didactic content is the development of the distinction between the sensations and the thoughts about the sensations.

**Week five: the body, the breath, and the environment.** The didactic content is the exploration of the interrelationship of the body, the breathing, the thoughts, and the feelings, and the beginning of the exploration of the obstacles to practice.

**Week six: the "choiceless" awareness.** The focus is on the observation of the internal and external world without the addition of meaning. The didactic content is the development of the capacity to tolerate discomfort and the examination of the patterns of reacting.

**Week seven: the development of a practice that suits the individual.** The didactic content is the identification and examination of the patterns that are most and least helpful, and the refinement of the individual practice.

**Week eight: the consolidation of the practice and the plan for the future.** The didactic content is the consolidation of what has been learned and the development of a plan for the continuation of the practice.

### 3.2 The practices

The formal practices are the body scan, the sitting meditation, and the mindful movement. The informal practices are the practice of bringing the attentional quality of mindfulness into the ordinary activities of the day — the "bring it back" practice, the practice of noticing the arising of thoughts and returning the attention to the body.

### 3.3 The didactics

The didactic content of the course is not decorative. It includes the stress response, the relaxation response, the reframing of the patterns of thought, the "decentering" and the "reperceiving" as a subject, and the examination of the patterns of relating to oneself and to others.

## 4. Mindfulness-Based Cognitive Therapy

### 4.1 The structure

Mindfulness-based cognitive therapy is an eight-week group programme, and its structure follows the stress reduction course with a number of modifications, most of which are diagnostic in character.

**Session one** adds an explicit formulation: the young person and the group identify their own pattern of depressive relapse — the situation in which the relapse is likely to occur, the thoughts that accompany it, the feelings, the behaviours, and the consequences. The formulation becomes the working document of the eight weeks.

**Sessions two to five** follow the stress reduction structure, with the addition of the cognitive elements in the didactic content.

**Session six** focuses explicitly on the depressive thoughts and on the recognition of the signs of relapse, and it is the session that most directly reflects the cognitive therapy inheritance.

**Sessions seven and eight** consolidate the practice and, crucially, develop a detailed **relapse prevention plan**: the personal warning signs, the internal coping strategies, the external coping strategies, the situations in which the mood is vulnerable, and the plan for the seeking of help. The relapse prevention plan is one of the most practically useful clinical documents in the entire field, and Appendix F provides the template.

### 4.2 The adaptation for depression

The adaptations of the standard mindfulness practice for depressed patients are specific and important.

The **body scan** in a depressed patient is often difficult, because the body sensations are unpleasant and the depressed patient has a strongly negative appraisal of bodily experience. The adaptation is to shorten it, to work with a narrower focus, and to be prepared for a great deal of resistance.

The **sitting practice** in a depressed patient is often experienced as intolerable, because the mind is highly active and the content is heavily negative. The adaptation is to work with a very brief practice, to anchor attention in a bodily location, and to accept the disturbance.

The **mindful movement** in a depressed patient is often the most accessible of the practices, and it is frequently the entry point.

The **relapse prevention plan** is the diagnostic heart of the therapy, and it is the element that the research has most clearly identified as important.

# 5. Mindfulness-Based Approaches with Children and Adolescents

## 5.1 The developmental fit

The developmental fit of the mindfulness-based approaches with young people is good, and the reasons are specific.

**The attentional focus is developmentally congruent.** The practices require the young person to attend to the present moment, which is precisely the developmental task that the adolescent is working on, and the evidence from the contemplative psychology literature is that the development of the attentional and metacognitive capacities that the practice requires continues well into the third decade.

**The practice is developmentally consistent with the somatic work.** The body scan, which is the practice most used with young people, connects directly to the somatic and the trauma-informed literatures, and the developmental research on the adolescent brain and on the body's representation of emotion is the same research that the somatic fields use.

**It does not require the young person to talk about their difficulties.** A practice that consists of sitting quietly and attending to the breath is accessible to a young person who cannot or will not describe what is wrong, and this is a considerable advantage with the young people who most need help.

**The relationship to the developmental task of self-regulation.** The developmental literature describes the adolescent's self-regulation as immature in the presence of arousal, and the practice is, in effect, a structured training in the regulation of attention and of the response to arousal.

## 5.2 The developmental modifications

**The duration of the practice.** The standard practices are 30 to 45 minutes, and this is far too long for a young person. The developmental adaptation is brief practices of 5 to 10 minutes, and the "stop, take a breath, notice what is happening" informal practice, which can be practised in a school corridor in ninety seconds.

**The language.** The didactic content must be translated into the young person's own terms, and the abstraction of the standard material — the stress response, the relaxation response, the reperceiving — is generally too much for a young person. The practice can be taught without the theory, and for a young person the practice is usually sufficient.

**The developmental modification of the body scan.** The body scan with a young person can be done standing, in a chair, at a desk, or in a very short form. It can be done with the eyes open, which many young people prefer. And the practice of attending to the body is developmentally appropriate and it is also the

practice that the trauma field uses, which means that a young person with a trauma history may have a specific and complicated relationship with it.

**The group.** The eight-week group format is developmentally appropriate for adolescents, and the developmental literature on the adolescent peer group is directly relevant: a group of peers who are practising the same thing is a very different proposition from a group of adults.

**The parents.** The parental involvement in the mindfulness-based approaches is more limited than in the behavioural and the dialectical approaches, and this is a genuine weakness. The parents of a young person with emotional difficulties frequently need to understand what is being done, and the literature on the family dimension of the mindfulness-based approaches is thin.

### 5.3 The specific cautions

**The trauma caution.** This is the most important caution in the chapter, and the field has been slow to state it clearly. A practice that asks a young person to attend to the present moment, to the body, and to the sensations is, for a young person with a trauma history, a practice that may be profoundly reactivating, and the somatic and trauma fields have been clear that such practices must be titrated, prepared, and paced, and that a young person with a dissociative presentation may find the interior attention genuinely frightening.

**The risk of the young person treating the practice as a task to be performed.** The practice will be performed competently and will produce a report of benefit, and the young person will have learned the observable form of mindfulness without the underlying shift. The practitioner's task is to attend to the young person's relationship to the practice, and to notice the difference between a young person who is attending and a young person who is performing.

**The risk of the practice replacing the work.** A young person who has learned to manage their distress with a brief attention practice may decline to talk about the difficult things. The practice is a tool, and the developmental and clinical requirement is that it be used in the service of the developmental task rather than in place of it.

**The risk of a secular practice with a religious frame.** For a young person with a religious or spiritual practice of their own, the mindfulness-based approach may be welcome, or may be experienced as an intrusion, or may be welcome but in a modified form that they would prefer. The practitioner should ask.

## 6. The Third-Generation Mindfulness and the Contemporary Position

The field has continued to develop, and several of the developments are relevant to this course.

**Mindfulness-integrated behavioural therapy** and other hybrid approaches combine the mindfulness principles with the behavioural and cognitive techniques, and the rationale is that the two are addressing the same problem from different directions.

**Mindfulness-based relapse prevention** adapts the relapse prevention plan for a range of conditions other than depression.

**The "brief" mindfulness interventions** have been developed for schools and for primary care, and the evidence for brief interventions is generally weaker than for the full courses.

**The contemplative psychology literature** has developed a substantial research base on the developmental trajectory of the attentional and metacognitive capacities, on the relationship between contemplative practice and the brain, and on the cross-cultural comparison of contemplative traditions, and it is a better foundation for the clinical work than the clinical literature itself.

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## Further Reading for This Chapter

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- Baer, R., (Ed.). (2014). *Mindfulness-based treatment for behavioral disorders*. Guilford. — The adaptation literature.
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- Britton, J. P., et al. (Eds.). (2021). *The science of mindfulness*. — A critical, methodologically attentive review.

## 7. Case Illustration: The Boy Who Was Not Worried Enough

**The referral.** Ravi, seventeen, is referred by a school counsellor because of "low mood and not engaging." Ravi's grades have fallen from excellent to adequate; he has stopped going to the gym, which he attended five times a week; he has been "flat" for two months. He has no history of treatment. He is quiet, polite, and gives short answers. He says: "It's fine. I'm just tired of everything."

**The first session.** The practitioner asks the standard questions. Ravi's answers are notable for their flatness and for the absence of the usual content. "Are you sleeping?" "Fine." "Eating?" "Fine." "What's the mood like?" "Fine." "What's been going on?" He shrugs. "Nothing, really."

The practitioner does not pursue the interview. She says: "Can I ask you something a bit strange? Would you be willing to close your eyes for about three minutes and just notice what's going on in your body? You don't have to do anything with it. Just notice."

Ravi looks doubtful and then agrees, and does it.

After two minutes he opens his eyes. "That was weird." "What was weird about it?" He thinks. "I could feel my face. I hadn't noticed my face for a while." "Anything else?" He hesitates. "My chest felt tight. And my hands." "What about your hands?" "They felt like they were buzzing."

**The formulation.** The practice has produced the first substantive data in the case. Ravi's flatness is not a symptom that is being concealed; it is a **narrowing of attention** so total that he is not in contact with his

own experience. He is not depressed in the way that a depressed person is; he is *absent* — a developmental and attentional state that is consistent with a young person who has been managing a difficulty by attending to everything except himself, and who has now run out of the ability to do even that.

**The course.** Ravi is offered the eight-week group. He asks whether it is "like meditation" and says that he tried a meditation app once and found it "boring and stupid." The practitioner says: "It is a bit like that for some people, and you are allowed to find it boring for the first three weeks and then tell me it is boring. The rule is that you turn up."

He comes to eight. He is a poor meditator, in the sense that his attention wanders constantly and he finds the body scan irritating. He says in week three: "I don't think my brain does this." The practitioner says: "Which part?" "The not-thinking part. I can do the noticing part, sort of. But there's a bit where you are supposed to not think, and I can't." "Do you want to know what the noticing part is for?"

**The teaching moment.** This is the pivot of the case, and the practitioner takes it. "I want you to notice something about yourself, over the next fortnight, and I want you to notice it without trying to change it. You have been in here three times and you have said 'fine' to almost everything I have asked you, and I do not think you are being rude or that you are lying. I think that something is going on where the questions stop and the noticing starts. And I think you have got so good at not-noticing that the noticing is the thing you have lost, and it is the only thing that is going to get you back."

**The informal practice.** The brief practice they agree on is this: three times a day, Ravi stops, puts his hand on his chest, and takes one breath, and notices one thing — one sensation, one thought, one feeling. He is told that it will feel like nothing. He says: "That's a low bar." "It's a very low bar. The point is not to achieve anything. The point is to stop."

**The developmental framing.** The practitioner also makes the developmental point explicitly, because it is what makes the work possible with a seventeen-year-old: "You are at the age where your brain is being rebuilt, and this is the part of the rebuild that goes wrong when the wiring for attention and for noticing is under-used. The thing I am asking you to practise is the thing your brain is best at learning at the moment, and it is the thing that will make the rest of it easier."

**The outcome.** Over the eight weeks, Ravi's flatness lifts. He does not become eloquent about his feelings; he becomes able to report them, in short, unimpressive sentences, which is a considerable achievement. He says in week six: "I feel sad, actually. Quite a lot. I didn't know." The practitioner says: "That is good news, although it does not feel like it." He says: "It is horrible." "It is. It is also the noticing part working."

He returns to the gym, at reduced frequency, and his grades recover partially. He does not return to "fine."

**The teaching point.** The intervention was a practice and a question, and the question was the important part: "Why do you think I am asking you to notice your face?" The most common error with a flat, shut-down young person is to treat the flatness as resistance and to push harder, and the second most common is to treat it as depression and to medicate it. The third possibility — that the flatness is an attentional state that has become automatic and that the first task is to restore contact with the interior — is the one that a mindfulness-informed practice makes available, and it is the one that Ravi's own experience confirmed.

## 8. Evidence and Research Support

**The evidence is strong for one specific population and one specific purpose, and much weaker elsewhere.**

## 8.1 The relapse prevention evidence

The evidence for mindfulness-based cognitive therapy in the prevention of depressive relapse in adults is the strongest in the mindfulness literature, and it is also among the stronger bodies of evidence in the whole field. The 1990 trial and its replications compared mindfulness-based cognitive therapy with the continuation of antidepressant medication in patients who had responded to medication, and found substantial reductions in relapse rates. Subsequent meta-analyses have confirmed the effect, with a relative reduction in relapse of approximately 40 to 50 per cent in the pooled samples.

The limitations of this evidence should be stated. The trials were conducted in adults, predominantly in specialist settings, and the comparator — the continuation of antidepressant medication — is a conservative and relatively strong comparator, so the effect is a genuine and important one. The treatment was delivered by therapists trained in both mindfulness and cognitive therapy, which is a high level of training, and the effect sizes in routine services are smaller.

## 8.2 The adolescent and child evidence

The evidence for the mindfulness-based approaches with young people is substantially weaker and should be handled with more care.

A substantial number of controlled trials have been conducted with children and adolescents, in school settings and in clinical settings, with conditions including anxiety, depression, ADHD, trauma symptoms, and behavioural difficulties. The meta-analyses report small-to-moderate effects, with the effects on anxiety and on depressive symptoms the most consistently reported. The methodological quality of the studies is generally moderate, and several systematic reviews have raised the same concerns: the small samples, the heterogeneity of the protocols, the short follow-up periods, the high risk of bias in the primary studies, and a substantial risk of publication bias.

Two specific cautions apply. The first is the **developer allegiance** problem, which is substantial in this literature, and the second is that the "mindfulness intervention" in the literature is a heterogeneous category, ranging from a three-session information leaflet on the physiology of stress to a full eight-week course in a specific format, and the pooling of these is methodologically problematic.

**What the evidence supports.** That structured mindfulness-based programmes can produce small-to-moderate improvements in anxiety and depressive symptoms in young people, that they are acceptable, that they can be delivered in schools, and that the mechanisms proposed — attentional control, decentering, and the reduction of reactivity — are measurable and are associated with outcome.

**What the evidence does not support.** Any claim that mindfulness is superior to established treatments. Any claim that it is a substitute for the evidence-based treatments for specific conditions. Any claim that the effect sizes observed in the research literature will be reproduced in a service, where the participants are not volunteers.

## 8.3 The comparison with the established treatments

The comparison with the cognitive-behavioural treatments is instructive. In the head-to-head trials in adults, the mindfulness-based cognitive therapy and the cognitive-behavioural therapies produce comparable outcomes on depression and anxiety, with the mindfulness-based approach sometimes showing an advantage in the follow-up and sometimes in the measures of well-being. The honest summary is that the two are broadly comparable, with different emphases, and that the mindfulness-based approach's distinctive contribution is the relapse prevention evidence, which the cognitive approach has not matched.

# 9. Strengths and Limitations of the Mindfulness-Based Approaches

**Strengths.** The evidence for the prevention of depressive relapse is strong and clinically important, and the therapy is the best-supported example of a treatment that works by changing the young person's relationship to their own experience rather than by changing the content of that experience. The practice is accessible, the taught skills are simple, and the brief informal practice is genuinely usable in a school day. The approach fits the somatic and the trauma fields, and the compatibility is real rather than fashionable. The developmental fit with adolescents is good, and the attentional training is well matched to the developmental period. And the practices are free of the requirement that the young person verbalise their difficulties, which is a considerable advantage with the young people who most need help.

**Limitations.** The evidence for the clinical applications with young people is weak, and the field's confident tone is considerably ahead of its data. The practice can be performed rather than learned, and a young person can produce the observable form of mindfulness indefinitely without any change. The contamination of the practice by the requirement to perform is a serious methodological problem for the whole literature. The approach is culturally marked, in a way that is discussed below and is more serious than is generally acknowledged. The engagement rate in the research is low, and the young people who are most in need are the ones least likely to volunteer for a meditation course. And the theory is thin, in comparison with most of the theories in this book, which is both its weakness and, arguably, the source of its clinical flexibility.

# 10. Cultural and Ethical Considerations

## 10.1 The cultural and religious dimension

The mindfulness-based approaches were developed in the West, in a clinical context, by a practitioner who had studied a specific Buddhist contemplative practice, and the literature has engaged with the origin of the practices in a variety of ways — from denial, through appropriation, to an increasingly sophisticated engagement with the question of what it means to take a practice out of its religious and ethical context and use it clinically.

The ethical issues are real and a student should be able to state them.

**The question of provenance.** A practice that is embedded in a religious tradition, with a cosmology, a community, a lineage, and a set of ethical commitments, is used in the clinical setting stripped of most of these. The clinical practitioner is rarely in a position to transmit the tradition, and the result is a technique without its frame, which is a legitimate concern and one that the secular mindfulness researchers have themselves raised.

**The question of cultural fit.** The clinical literature on the cultural adaptation of mindfulness interventions with minority populations is small but growing, and the studies that exist have found that the adaptations that are necessary are substantial: the language, the metaphors, the practice's relationship to the body, the family's involvement, the framing of the difficulty, and the relationship between the practice and the young person's own spiritual or cultural resources. A practice that is delivered without adaptation and that is understood by the young person as a foreign practice of a different kind of religion is unlikely to be taken up.

**The question of the young person's own framework.** A young person who has a religious practice of their own — which is a large number of young people globally — may find the mindfulness approach either compatible and clarifying, or a secular alternative they have been taught to distrust, or a valuable secular technique that they can use alongside their religious practice. The practitioner's responsibility is to ask, and the clinical literature on this is very thin.

**The question of the body.** The practices are, in their standard form, practices of interior attention, and this is a substantial cultural variable. In a context in which the body is understood as a site of danger, of impurity, or of the inappropriate, or in which the interior life is understood to be accessible only through a spiritual authority, the practice may be inappropriate. And in a young person with a trauma history, the interior attention is specifically reactivating, as Section 5.3 stated.

## 10.2 The ethical requirements

**Informed consent must include the provenance.** A young person and their parents should be told that the practice comes from a contemplative tradition, and should be told what has been changed and what has not. A practice that is presented as a neutral, secular technique when its provenance is a specific religious tradition is a misrepresentation, and a misrepresentation in a domain in which the young person and their family may hold strong commitments.

**The trauma-related requirements.** The preparation, the titration, and the monitoring of the young person's response are ethical requirements and not merely technical preferences, and the somatic and trauma-informed literature is clear that interior attention practices with a young person who has a trauma history require preparation, monitoring, and the availability of alternative grounding.

**The requirement not to substitute the practice for the work.** The practice must be used in the service of the young person's developmental task and of the relationships and situations in which the difficulty is maintained, and the practitioner must be alert to the young person who has found a way to use the practice as a way of not engaging.

**The equity requirement.** The practice is a practice of attention, and attention is a resource. A service that offers it to the young people who can attend an eight-week group and who can afford the associated time and transport is delivering a treatment that is systematically unavailable to the young people with the greatest need. This is the equity argument that Chapter 50 develops, and it applies here with particular force because the intervention is group-based and time-intensive.

**The ethical value.** The ethical value of the approach should also be recorded. It is a treatment that offers a young person a relationship with their own experience that does not require an adult's interpretation, that does not require them to be able to explain themselves, and that treats their attention as their own and not as a deficit. That is a considerable offer to a young person who has been told all their life what is wrong with them.

## 11. Key Terms

### Key Terms at a Glance

- **Mindfulness** — Attention to the present moment with a non-judgemental quality, together with an orientation of the self toward the present moment; the definition developed by the contemporary researchers.
- **Attentional framework** — The account of mindfulness as comprising attention and orientation, which locates the practice as a change in the orientation of the self rather than merely as a change in the content of attention.
- **Decentering** — The capacity to perceive a thought or feeling as a mental event rather than as a fact, and the capacity to reduce its believability or impact.
- **Reperceiving** — The capacity to see a situation from a range of perspectives, including the perspective of the wider system and of one's own history; a trainable capacity and a mechanism of change.
- **Reactivity** — The conditioned tendency to respond to stimuli in a habitual way; the mechanism that mindfulness reduces.
- **Response flexibility** — The capacity to respond appropriately rather than habitually.
- **Mindfulness with acceptance** — The capacity to be present with an unpleasant experience without suppressing it or reacting to it.
- **The eight mechanisms** — Attentional control, attentional awareness, decentering, reperceiving, reduction of reactivity, response flexibility, mindfulness with acceptance, and values-based action.
- **Mindfulness-based stress reduction (MBSR)** — Kabat-Zinn's eight-week course, developed in a medical clinic, with the body scan, sitting meditation, and mindful movement as its formal practices.
- **The body scan** — A 45-minute practice of attending to the body from the feet to the head; the practice most frequently adapted for work with young people.
- **Mindful movement** — A practice of gentle, deliberate movement with the attention on the sensations; frequently the most accessible practice for a young person.
- **Mindfulness-based cognitive therapy (MBCT)** — The eight-week group programme developed for the prevention of depressive relapse, combining mindfulness practice with the cognitive model and with a personalised formulation.
- **The depressive relapse formulation** — The session-one working document identifying the young person's pattern of relapse: the situation, the thoughts, the feelings, the behaviours, and the consequences.

**Key Terms at a Glance (continued)**

- **The relapse prevention plan** — The structured document specifying the personal warning signs, the internal and external coping strategies, the vulnerable situations, and the plan for seeking help; reproduced in Appendix F.
- **Brief informal practice** — A short, in-the-moment attentional practice, adaptable to a few minutes and usable in a school day.
- **Contemplative psychology** — The research literature on the developmental and neurological correlates of contemplative practice, and the better empirical foundation for the clinical work.
- **Adapted mindfulness** — The culturally and developmentally modified delivery of the practices, with modification of language, metaphors, the relation to the body, the family's involvement, and the framing of the difficulty.
- **Attention as a resource** — The recognition that the capacity to attend is unevenly distributed, and that an intervention which trains it is therefore an equity issue.

## 12. Chapter Summary

Mindfulness entered clinical psychology in the late 1970s through the stress reduction clinic at the Massachusetts General Hospital, and its clinical proposition — that a person trained to attend to the present moment is less at the mercy of the reactivity that maintains distress — was for a long time unfashionable. Mindfulness-based cognitive therapy was developed in the 1990s for one purpose, the prevention of depressive relapse, and the randomised trial that established it is one of the very few studies in the field that produced a large effect on a clinically important outcome.

The mechanism the tradition proposes is the development of decentring, the capacity to observe thoughts and feelings as events in the mind rather than as facts about the world, together with reperceiving, the capacity to see a situation from a range of perspectives. The contemporary research literature has identified a set of eight mechanisms, of which attentional control, decentering, and the reduction of reactivity are the most directly relevant to work with young people.

The stress reduction course is an eight-week course with a body scan, a sitting meditation, and a mindful movement as its formal practices and a specific didactic content; the cognitive therapy course adds the depressive relapse formulation in session one, the recognition of relapse signs in session six, and the relapse prevention plan in the final session, which is one of the most practically useful clinical documents in the field.

The developmental fit with young people is good — the practice is developmentally congruent, it connects to the somatic work, it does not require the young person to talk, and it matches the adolescent's developmental task of self-regulation — and the modifications are concrete: brief practices, translated language, the body scan adapted to standing or seated, and the peer group. The cautions are equally concrete, and the most important is the trauma caution, which the clinical field has been slow to state.

The evidence is strong for the relapse prevention application and much weaker for the clinical applications with young people, where the meta-analyses report small-to-moderate effects in studies of generally

moderate quality with substantial allegiance and publication bias. The cultural and religious provenance of the practice is a real ethical question that the field has engaged with inadequately, and the case of Ravi shows the practice doing what it does best, which is to give a young person who has become absent from his own experience a way back in.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the origin of the mindfulness-based stress reduction course and the clinical observation that produced it.
2. State the attention and orientation components of the definition of mindfulness and explain the significance of the non-judgemental quality.
3. Define decentering and contrast it with cognitive fusion.
4. State the eight mechanisms and identify which three are most relevant to developmental work.
5. Describe the structure of the eight-week course, including the formal practices and the didactics.
6. Describe the three modifications that mindfulness-based cognitive therapy makes to the stress reduction course and state the significance of each.
7. Explain the adaptive of the body scan, the sitting practice, and the mindful movement for a depressed adolescent.
8. State the developmental modifications required for the mindfulness-based approaches with young people.
9. State the trauma-related caution and the four requirements that follow from it.
10. Summarise the evidence for the mindfulness-based approaches with young people, and state three methodological concerns about it.

## 14. Practice Activities

**Practice and Application**

### Activity 22.1 — The Body Scan (30 minutes, individual, weekly)

Practise the body scan every day for a fortnight, in the standard form or a shortened form, and keep a brief note of what you notice and what you find difficult. The purpose is not to achieve a state but to become familiar with the practice as it is actually done rather than as it is described. The notes are the useful artefact, particularly the notes on what you avoided.

### Activity 22.2 — The Relapse Prevention Plan (90 minutes, individual, written)

Write your own relapse prevention plan using the template in Appendix F: the warning signs, the internal coping strategies, the external coping strategies, the vulnerable situations, and the plan for seeking help. Then, in pairs, read one another's plans and give the feedback you would give a young person. The exercise produces a document that most students find surprisingly useful and that they continue to use, and the pairing produces the feedback about clarity that the individual exercise cannot.

### Activity 22.3 — The Provenance Conversation (75 minutes, pairs)

In pairs, one student plays a sixteen-year-old from a family for whom the family's religious practice is central and who has been sent to an eight-week mindfulness course by a school counsellor, and the other plays the counsellor. The counsellor's task is to explain the provenance, the adaptation, and the alternatives without implying that the family's practice is inferior. The exercise is the most important preparation for the cultural work in Chapter 48, and it is much harder to do than it looks.

## 15. Annotated Reading Pathways

**For the practice.** Read Kabat-Zinn (1990) for the course and Segal, Williams, and Teasdale (2002) for the relapse prevention work, and then Williams, Teasdale, and Segal (2014), which is the most usable of the three with young people.

**For the evidence.** Read the Goyal and colleagues meta-analysis, and then read one of the critical reviews of the mindfulness literature with attention to its methodological criticisms. Then read the adolescent systematic reviews with the same attention. The contrast between the confident public accounts and the actual state of the evidence is the most important thing a student can learn from this chapter.

**For the mechanisms.** Read the research literature on the eight mechanisms, and read the contemplative psychology literature on the developmental trajectory of attention and metacognition. The mechanisms are the substance of the therapy, and the developmental literature on them is the most practically useful part.

**For the cultural and ethical questions.** Read the literature on the adaptation of mindfulness for minority populations and on the provenance question, and read the somatic and trauma-informed literature on interior attention with young people who have been abused. The two together constitute the most careful preparation a practitioner can have for this work.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Mindfulness-Based Cognitive Therapy and Mindfulness Approaches** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is mBSR, MBCT, and the cultivation of attentional flexibility in young people. In adolescent work, understanding the model is

only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The mechanism to keep in view is how thoughts, bodily states, avoidance, reinforcement, and action maintain one another in repeating cycles. The characteristic professional risk is arguing with a young person, overlooking development and environment, or treating a thought as false merely because it is painful. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in structured sessions, homework reviews, school-based practice, and family-supported skill building, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased

self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tobias is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tobias's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tobias explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on appraisal, belief, emotion, behaviour, skills practice, and relapse prevention. The first intervention is deliberately modest. Tobias chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tobias has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tobias can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan

that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or

term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Mindfulness-Based Cognitive Therapy and Mindfulness Approaches

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Mindfulness-Based Cognitive Therapy and Mindfulness Approaches** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Mindfulness-Based Cognitive Therapy and Mindfulness Approaches** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Mindfulness-Based Cognitive Therapy and Mindfulness Approaches**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Mindfulness-Based Cognitive Therapy and Mindfulness Approaches to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART IV

# Behavioral Theories

The behavioral tradition: learning as the engine of change, the systematic analysis of behaviour, and the translation of learning principles into interventions that can be measured, replicated, and evaluated.

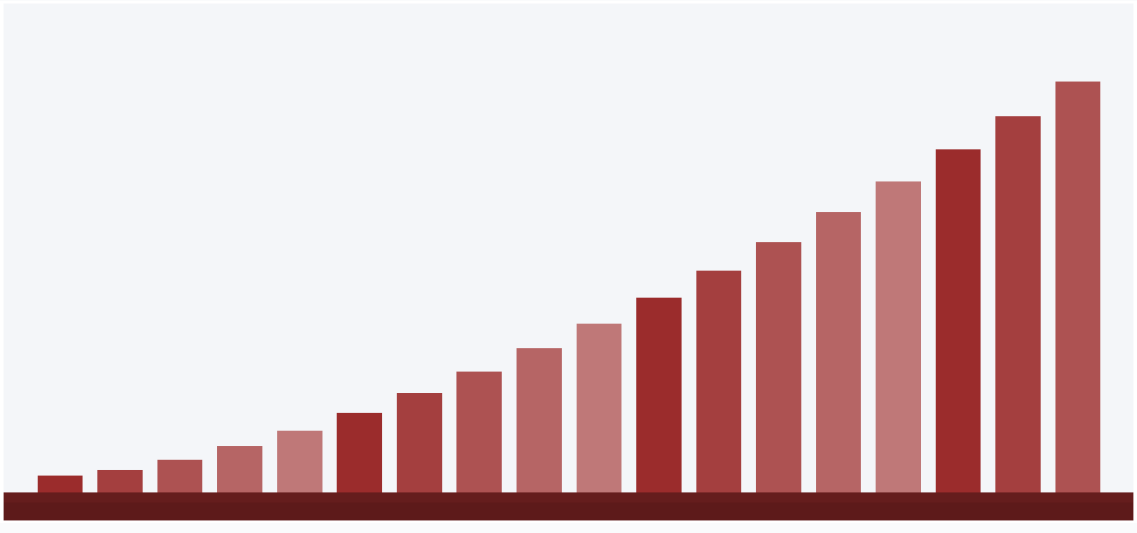
CHAPTERS IN THIS PART


23 Behavior Therapy: Principles of Learning
24 Applied Behavior Analysis: Functional Assessment
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27 Behavioral Activation and Activity Scheduling
28 Contingency Management in Schools and Clinics
29 Functional Analysis of Self-Injurious and Aggressive Behaviour

CHAPTER 23

# Behavior Therapy: Principles of Learning

Classical and operant conditioning, the law of effect, and the epistemology of observable behaviour.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the historical emergence of behaviour therapy from the psychology of learning, tracing the work of Pavlov, Thorndike, Watson, and Skinner.
- State the epistemological commitments of radical behaviourism and explain both their power and their limitations for clinical work.
- Describe the general procedure of classical conditioning, and the phenomena of acquisition, extinction, spontaneous recovery, generalisation, and discrimination.
- Distinguish the four operant contingencies, and distinguish negative reinforcement from punishment in the terms the literature actually uses.
- Explain the law of effect, and critically discuss the problem of establishing the reinforcing value of a stimulus.
- Describe shaping, chaining, and prompting as behavioural technologies, and specify the circumstances in which each is indicated.
- Explain the methods used to measure behaviour, and why the measurement question is not a technicality.
- Apply the principles of learning to a case involving a young person, and identify where the learning account is incomplete.

## Chapter Overview

Every therapy in this book contains an implicit theory of how people change. The behavioural theories state theirs most plainly, and that plainness is both their greatest strength and the source of most of the misunderstandings that surround them.

The behavioural account begins with a claim about evidence. Human beings are complex, but the complexity is not a reason for mystery. Behaviour is a function of conditions; conditions can be specified; specified conditions can be altered; and altered conditions produce altered behaviour. The chain from condition to consequence to behaviour is the causal spine of the tradition, and almost every technique in the behavioural repertoire is a way of arranging that chain deliberately.

The account begins in the laboratory, and it is important to be accurate about what the laboratory actually established. Pavlov showed that a neutral stimulus could acquire the capacity to elicit a response through pairing with a stimulus that already elicited it. Thorndike showed that the strength of a habit increased with its satisfying consequences and decreased with its annoying ones. Watson converted both findings into a methodological manifesto about the irrelevance of consciousness. Skinner extended the operant analysis to voluntary behaviour and made the analysis of behaviour the organising principle of a whole discipline, including the analysis of language, thought, and self-control.

What the laboratory did not establish is that a simple and universal set of principles explains all human action. Behavioural therapists work with children who have been abused, who are grieving, who are autistic, who are in families where the contingencies are not merely misarranged but actively hostile. The learning account is a powerful tool for understanding a great deal of that, and it is not a sufficient account of any of it. A student who learns to hold both of these statements at once is doing something the discipline itself has found difficult.

This chapter presents the principles in their original and technical form, because the clinical techniques of the following chapters cannot be understood without them. It then examines the epistemological position from which the principles were derived, because the position is genuinely powerful and genuinely constraining, and a practitioner needs to know both.

# 1. Historical Context

## 1.1 The reflex arc and the era of the reflex

The behavioural tradition inherited its basic unit of analysis from the physiology of the nervous system. In the decades before the First World War, psychology in Europe was dominated by a set of questions about the reflex: what counts as a reflex, how is a reflex arc organised, what is the difference between a voluntary and an involuntary response? The Russian physiologists, following Sechenov, Pavlov, and Vvedensky, treated the nervous system as a machine for transforming stimuli, and the psychologist's task as the description of that transformation.

Two features of this inheritance are worth noticing, because they shaped everything that followed. The first is the commitment to a mechanistic vocabulary: *stimulus*, *response*, *reflex*, *conditioned*. The second is the treatment of the organism as something that is acted upon. The reflex arc was an incoming channel, and the research programme of the early twentieth century was to trace the channels.

This inheritance supplied behavioural psychology with a vocabulary and a method, but it also supplied a limitation that would take fifty years to recognise. If behaviour is a chain of responses to stimuli, then the individual is passive, and there is nowhere in the picture for intention, meaning, or choice.

## 1.2 Pavlov and the conditioned reflex

Ivan Pavlov was a physiologist of digestion. His celebrated work on the conditioned reflex began, by his own account, as a methodological annoyance: he was measuring salivary secretion in dogs and noticed that the dogs began secreting before the food arrived, in response to the sound of the metronome or the sight of the experimenter. Rather than treating this as an inconvenient artefact, Pavlov made it the object of study, and the "physiological" method of the conditioned reflex became the paradigm for the experimental analysis of learning.

The core demonstration is now so familiar that it is easy to underestimate how unusual it was. A bell produces no salivation. Food produces salivation. Pair the two, and after a number of pairings the bell alone produces salivation. The new stimulus has acquired the capacity of the old one.

Pavlov's work established the *laws* of conditioning: acquisition, extinction, spontaneous recovery, generalisation, discrimination, and the phenomena of higher-order conditioning and inhibition. These laws are the raw material of every behavioural therapy in the field, including the exposure therapies of Chapter 25 and the systematic desensitisation of the fear that the account of classical conditioning was originally designed to explain.

Pavlov also made a methodological move that matters enormously: he insisted on analysing the *whole* nervous response, including the orienting response and the emotional and vegetative components, and not only the salivation. His conceptual nervous system was a system of stimulus and response relations, and his theory of conditioned inhibition in particular anticipated, in a different language, a great deal of what the

cognitive and mindfulness traditions later described as the inhibition of a learned response by a competing one.

## 1.3 Thorndike and the law of effect

Edward Thorndike began with a puzzle box: a cat that had to step on a pedal to escape, and that, on being placed in the box, took a long time to escape by accident. Thorndike timed the escape and recorded the curve, and the curve became the first learning curve in psychology. It rose steeply at first and then flattened, and Thorndike's explanation of the flattening was the law of effect:

> *Of several responses made to the same situation, those accompanied or closely followed by satisfaction to the animal will, other things equal, be more firmly connected with the situation, so that, when it recurs, they will be more likely to recur; those accompanied or closely followed by discomfort will, other things equal, be less firmly connected with the situation, so that, when it recurs, they will be less likely to recur.*

Three features of the formulation matter. It is hedged ("other things equal"), which is a technical admission that other variables matter. It is about the *connection* between a situation and a response, which is a claim about selection rather than about instruction. And it is empirical rather than mentalistic, because Thorndike was explicit that his experiments contained no consciousness, and the law of effect was offered as a replacement for the appeal to consciousness that he found useless.

Thorndike's curriculum is also the origin of the *law of readiness* and the *law of exercise*, both of which appear later in this book in contemporary form.

## 1.4 Watson and the behavioural manifesto

John Broadus Watson's 1913 paper, "Psychology as the Behaviorist Views It," was the founding statement of the school, and it is worth quoting its most famous sentence in full, because the sentence is more radical than it usually appears:

> *Psychology as the behaviorist views it is a purely objective experimental branch of natural science. Its theoretical goal is the prediction and control of behavior. Introspection forms no essential part of this method, for the reliability of introspection is secured only by those who have trained themselves to use it in accordance with the very conditions under which it is employed.*

Watson was not claiming that inner experience does not exist. He was claiming that it cannot function as a datum in a science of behaviour, because the conditions under which it is reported are not subject to the controls that make other data reliable. The methodological proposal, in other words, was a proposal about what counted as evidence.

This is the point at which behaviour therapy inherited a characteristic and still controversial stance: that the practitioner's data is what can be observed and counted, and that reports of internal states are data only under specified conditions. Much of what follows in this book — the cognitive, the humanistic, the existential traditions — exists in part as a response to this stance, and the arguments between them are more interesting when the student understands that they are partly arguments about epistemology.

### 1.5 Skinner and the functional analysis of behaviour

Burrhus Frederic Skinner's work from the 1930s onwards was the decisive extension of the tradition, and it transformed it in two directions.

The first was the extension from reflexes to *operant* behaviour, that is, to behaviour that is emitted rather than elicited. Skinner demonstrated that behaviour is shaped by its consequences, and that the analysis of a behaviour is the analysis of the contingencies that produce and maintain it.

The second was the extension into a full *methodology*. Skinner argued that the study of behaviour was not an exotic specialism of psychology but the fundamental discipline, from which psychology itself was a branch, and he insisted that the analysing of behaviour required no appeal to hypothetical inner causes. His 1953 book *Science and Human Behavior* set out the system; his 1957 *Verbal Behavior* extended it to language; and his later work on *Schedules of Reinforcement* (with Ferster) and on the analysis of private events developed the experimental apparatus that became the therapy.

Skinner's methodological claim was the important one, and it is often caricatured. He did not say that inner events do not occur. He said that inner events are behaviour, that is, they are subject to the same analysis, and that when one is describing behaviour one should describe the conditions under which the event occurs and the consequences that follow it, rather than positing a private cause that explains the observable. A private event is defined by its consequences.

### 1.6 The later developments

The period from the 1960s to the present has seen the behavioural tradition absorb, and then sometimes reject, a series of additions:

- **Mediated learning** (Bandura, Rotter) added the concept of expectancy and self-efficacy between stimulus and response. The social learning theory of 1977 argued that most human learning occurs without direct reinforcement, through observation and modelling. This is the single most important development for work with young people, because it means that a child can learn a behaviour from watching a peer, and that learning can be arranged in a classroom without waiting for the child's own behaviour to be shaped.
- **The cognitive revolution** (Lazarus, Beck, Ellis) returned to the interior, but in a form that the behavioural tradition could partly accommodate: attention, memory, and belief are treated as mediational processes with their own contingencies, and the thought record is best understood as a data-collection procedure.
- **The contextual tradition** (Bowers, Goldstein, Heiman, Jacobson) restored the importance of context and of the client's frame of reference, and produced a body of research on the language of therapy that is directly relevant to the cultural considerations in Chapter 48.
- **Applied behaviour analysis** (Baer, Wolf, Risley, and the applied literature) institutionalised the methodological standards of single-case experimental designs, functional analysis, and behaviourally defined treatment targets, and it is the applied wing that produced the functional assessment model in Chapter 24.

## 2. The Epistemology of Observable Behaviour

## 2.1 The radical behaviourism of Skinner

Skinner's position is usually described by the formula that behaviour is a function of the organism and its environment, and that the environment selects behaviour. Both halves of that formula carry weight.

The first half — the functional relation — asserts that behaviour is not caused by something hidden; it is a *function of* specified conditions. A functional relation is not a cause in the ordinary sense. It is a statement of covariation under specified controls. If a behaviour varies systematically with a specified environmental condition, and the condition is experimentally manipulated, the relation can be used to predict and to control. The precision of the claim is what makes it useful, and the modesty of it is what makes it defensible.

The second half — selection by consequences — asserts that the environment does not merely trigger behaviour but *selects* it, in the sense that those behaviours that produce consequences that the organism wants to repeat are more likely to be emitted in the future. The metaphor of selection replaced the metaphor of release, and it displaced a long tradition in which behaviour was thought of as something that issued from within and was then released by a stimulus.

## 2.2 Public and private events

Skinner's most consequential analytic move was the treatment of private events. A private event — a thought, a feeling, an expectation, an intention — is defined not by its phenomenology but by its discriminative stimulus and its consequences. "Anger" is not a cause; anger is a pattern of behaviour (in the broad sense, including verbal behaviour) that is evoked by particular stimuli and that has particular consequences. The clinician's task is to specify the antecedent conditions and the consequences, and to find the functional relation.

The analytic value of this is considerable, because it makes the private event available for prediction and control in exactly the way that public events are. Its philosophical cost is considerable too, because it appears to reduce the person to a black box that is opened only at the input and output ends. Almost all the major criticisms of behaviour therapy — from humanistic, existential, feminist, and cultural perspectives — are in some form a criticism of this position.

## 2.3 The limits of the epistemological position

A student should be able to state three limits without caricature.

**First**, the account is descriptively complete for a great deal of behaviour and explanatorily thin for a great deal more. The question "why did she refuse to come to school?" is easy to answer behaviourally and unhelpfully so. The behavioural answer is a set of contingencies. The question "what would it take for her to come?" is the answer the behavioural account gives best, and it is a much better question than the first.

**Second**, the account under-specifies the *meaning* of consequences. Whether a given consequence is a reinforcer is not a property of the consequence but of the relation between the consequence and the organism's current state, and there is a circularity in the analysis that behaviour analysts have addressed through a variety of techniques, none of which is entirely satisfying.

**Third**, the account struggles with the phenomena of trauma, loss, and attachment, where the clinically relevant events have no current contingency that would maintain them. A child who does not speak after a year of silence at school is not maintained by a contingency in the present. The behavioural account of such a case is not wrong, but it is radically incomplete, and this incompleteness is the reason that the

trauma-informed chapters in Part VI exist.

# 3. Classical Conditioning

## 3.1 The general procedure

In the general procedure, a previously neutral stimulus (the **conditioned stimulus**, or CS) is repeatedly presented with a stimulus that naturally produces a reflexive response (the **unconditioned stimulus**, or US), and the response that the US produces becomes the **unconditioned response** (UR). After pairing, the CS alone elicits a similar response, which is the **conditioned response** (CR).

The technical vocabulary matters because the therapy that follows from it uses it literally. In the treatment of a child's fear of the school canteen, the canteen is the CS, the pain of the allergic reaction is the US, the nausea is the UR, and the anticipatory nausea at the sight of the canteen is the CR. The aim of the behavioural treatment is to extinguish the CR while ensuring the US never occurs, which is the logic of counterconditioning and, in its more intensive form, the exposure work of Chapter 25.

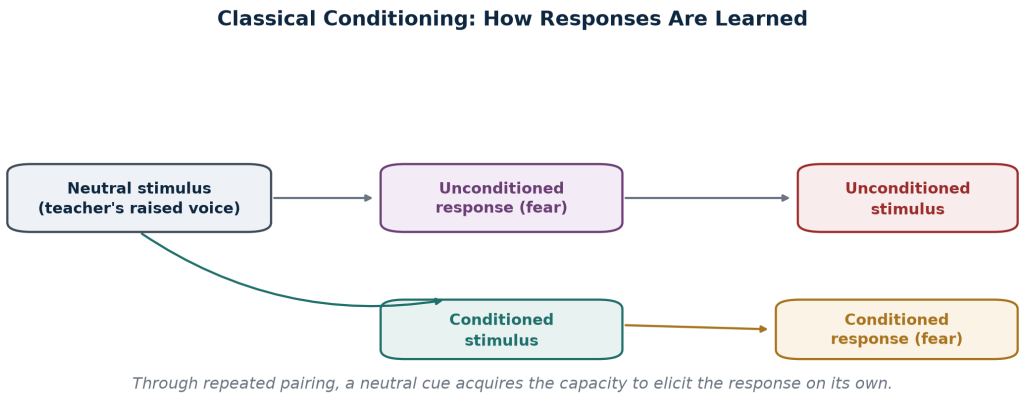



**Figure 23.11** The classical conditioning procedure, showing the pairing of a neutral stimulus with an unconditioned stimulus and the acquisition of a conditioned response.

## 3.2 Acquisition, extinction, and spontaneous recovery

**Acquisition** is the phase in which the CR is being learned. In most human fear conditioning, acquisition is rapid: one or two pairings suffice to establish a substantial conditional response, and much of the apparent power of phobias comes from the fact that a single traumatic pairing can be enough.

**Extinction** is the phase in which the CS is presented repeatedly without the US. The CR declines. Importantly, extinction does not destroy the conditioned connection; it inhibits it, and the original learning is masked by new learning. This is the basis of the phenomenon of **spontaneous recovery** — the reappearance of the CR after a rest interval, sometimes to a substantial proportion of its original strength, without any further pairing.

**Spontaneous recovery** has direct clinical significance. It explains why a young person's fear may return at full strength after a holiday, after an illness, or after a gap between sessions, and why a single successful exposure is not evidence of cure. It is also the reason that the treatment of fear disorders requires *retrieval* work — repeated re-exposure over time — rather than a single in-session success.

**Reinstatement** is the third recovery phenomenon and the most clinically alarming: after extinction, presenting the US once again (a single painful event) can restore the extinguished CR to full strength. A young person whose allergy-related fear has been treated and who then has one severe reaction in the canteen is a reinstatement case, and the clinician should anticipate it and prepare for it.

## 3.3 Generalisation and discrimination

**Generalisation** is the tendency for a response conditioned to one stimulus to be elicited by similar stimuli. A child conditioned to fear the school dentist's chair may fear all chairs, all white coats, all waiting rooms. Generalisation is the reason a single specific fear can organise a wide and generalised pattern of avoidance, and it is the reason that a treatment which only targets the original stimulus may leave the pattern intact.

**Discrimination** is the inverse process, in which the organism learns to respond to one stimulus and not to the others, and it is learned through differential reinforcement. Clinically, discrimination training is used both to *narrow* an over-generalised fear (so that the child responds to the specific stimulus only) and to *broaden* an under-discriminated pattern of behaviour, in which a child cannot tell which of a set of adult instructions is being given.

Generalisation and discrimination are in a functional relation to each other that parallels the relation between the cognitive and behavioural traditions: generalisation supports flexibility but also pathology; discrimination supports precision but also rigidity. Clinical work usually involves moving in both directions at once.

## 3.4 Higher-order and second-order conditioning

A CS can itself be paired with a second neutral stimulus, which then acquires the capacity to elicit the CR. This is **second-order conditioning**, and it is how symbolic and verbal fears arise: a child who is frightened by an animal develops a fear of the word for the animal because the word was presented in the presence of the animal.

Second-order conditioning is of considerable importance with young people because it means that the range of stimuli that can maintain a fear is much larger than the range of stimuli that were directly paired with the aversive event. In a school, a pupil who has been teased about an interest may be conditioned to fear not only the teasing but the word used for the interest, the room in which the teasing occurred, and the appearance of the pupils who did the teasing. The clinician who treats only the original incident will find the fear returning.

## 3.5 Inhibition, overshadowing, and blocking

**Conditioned inhibition** arises when a stimulus is repeatedly presented in the absence of the US, so that it comes to signal safety. In treatment terms, a stimulus that reliably accompanies safety can be used to inhibit fear, and this is the theoretical basis for the "safety signal" component of many exposure-based protocols and for the use of a trusted adult as a co-therapist in the treatment of a young person's avoidance.

Two other phenomena have direct relevance to treatment design:

- **Overshadowing** occurs when a more informative stimulus in a compound comes to dominate learning. In treatment terms, if a young person is exposed to a feared stimulus in the presence of a highly salient alternative stimulus — a crying sibling, a looming deadline, an argument between parents — the

alternative will capture the learning, and the exposure will fail to generalise. The practical implication is that exposure must be designed so that the target stimulus is the most informative element in the situation.

- **Blocking** occurs when a prior CS has already been learned, so that a new stimulus in the same compound is blocked from acquiring associative strength. Clinically, blocking explains why a second fear may not be learned, and why a young person with a well-established fear of one thing may appear unmoved by a second aversive experience. It also explains an apparent clinical paradox: a person who is already very anxious may not learn a new fear because the existing learning blocks it.

### 3.6 Counterconditioning

**Counterconditioning** is the procedure in which a CS that has acquired aversive properties through pairing with a US is paired with a stimulus that produces an incompatible response, so that the CS comes to evoke the new response instead. The classic example pairs a fear stimulus with relaxation.

The technique has two historical clinical forms. In **systematic desensitisation** (Wolpe, 1958), the patient is taught to relax, constructs a hierarchy of anxiety-provoking images, and pairs each image with relaxation while moving up the hierarchy. In **aversion conditioning** (also Wolpe), the same principle is used in reverse, pairing an undesirable behaviour with a stimulus that produces an aversive response.

The two forms have sharply divergent evidence bases. Systematic desensitisation has a long tradition of controlled studies and remains a component of many established protocols. Aversion conditioning is used far more sparingly, and its modern use is largely confined to specific applications (for example, in the treatment of specific paraphilias) under tightly controlled conditions. A student who reads Wolpe's original case material without this distinction will badly overestimate the clinical acceptability of aversive techniques with young people, and Chapter 25 returns to the topic with the contemporary evidence and the safeguarding constraints.

## 4. Operant Conditioning

### 4.1 The four contingencies

Operant behaviour is behaviour that operates on the environment and is selected by its consequences. The basic analysis distinguishes four contingencies, and the distinction is defined by two questions: what happens *before* the behaviour, and what happens *after* it.

| Contingency | Consequence | Effect on behaviour |
|---|---|---|
| Positive reinforcement | A stimulus is added after the behaviour | Behaviour increases |
| Negative reinforcement | A stimulus is removed after the behaviour | Behaviour increases |
| Positive punishment | A stimulus is added after the behaviour | Behaviour decreases |
| Negative punishment | A stimulus is removed after the behaviour | Behaviour decreases |

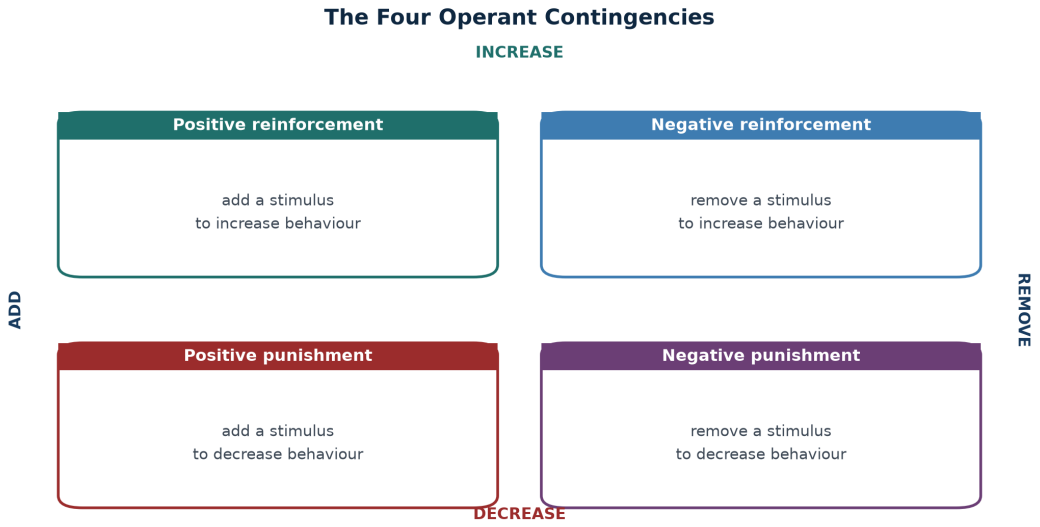



**Figure 23.12** The four operant contingencies arranged in a two-by-two matrix by whether a stimulus is added or removed and whether the behaviour increases or decreases.

The most consequential error in applied practice is the confusion of negative reinforcement with punishment. **Negative reinforcement** increases behaviour: it is the removal of an aversive stimulus contingent on a behaviour, as when a young person is permitted to leave the classroom after saying that they are being bullied. **Negative punishment** decreases behaviour: it is the removal of a valued stimulus contingent on a behaviour, as when a teenager loses phone access after an outburst.

The clarifying statement that a student should be able to make without hesitation is this: reinforcement and punishment are defined by their *effect on the future frequency of the behaviour*, and positive and negative are defined by whether something is added or removed. Nothing in the terminology refers to anything being good or bad.

## 4.2 Reinforcement

A **reinforcer** is a stimulus that, when made contingent on a behaviour, increases the future frequency of that behaviour. Four properties distinguish reinforcers, and the differentiation is practically important in the design of any behavioural intervention.

**Value** is the degree of preference the organism shows for the stimulus, which determines the amount of behaviour that can be acquired and maintained by its delivery. A young person's preference ordering is not fixed and cannot be assumed from developmental stereotypes; it must be established empirically, which in practice means asking the young person what they want, and taking the answer seriously.

**Deprivation** refers to the organism's current state with respect to the stimulus. A reinforcer that is abundant has little value. The clinical significance for adolescents is direct: the substances and behaviours that concern us most are, in the language of the framework, highly valued and reliably deprived, and the most successful programmes for these behaviours are typically those that offer something of competing value.

**Number of and size of reinforcers** determine how much behaviour can be acquired per unit of effort. A small reinforcer delivered frequently can shape behaviour effectively; a large one delivered rarely cannot

sustain a dense reinforcement schedule.

**Immediacy** of the consequence is critical. The closer in time the consequence follows the behaviour, the stronger the association. This is why a token system, with its immediate small reinforcers and a delayed backup reinforcer, works considerably better in practice than an abstract promise of a reward next week — and it is one of the strongest arguments for the concrete and immediate in behavioural work with young people.

**Motivating operations** are events that establish or abolish the reinforcing effectiveness of a stimulus. A young person who has been standing outside in the cold for an hour has a high motivating operation for the reinforcer of going inside, and the value of that reinforcer will decline as the young person is warmed. A behaviour that is reinforced while a motivating operation is in effect, and is then left in place after the operation is abolished, will extinguish quickly. This is the mechanism behind a very common practical failure: a behaviour that was effectively reinforced in the school corridor stops being maintained once the underlying need is met, and the young person is then described as "non-compliant."

## 4.3 Punishment

The term carries strong connotations and the literature uses it technically. Four distinctions are essential.

**Positive punishment** is the application of an aversive stimulus contingent on a behaviour. In practice it is used sparingly, and where it is used it is applied immediately, briefly, and with a clear functional purpose.

**Negative punishment**, also called *response cost* or *time-out from reinforcement*, is the removal of a valued stimulus contingent on a behaviour. It is far more common than positive punishment, and its most widely used forms in clinical settings — time-out, response cost, loss of privileges — are the ones that most often produce a *temporary* suppression of behaviour accompanied by an increase in escalation or aggression, because the removal is experienced as arbitrary and the behaviour becomes a protest rather than a choice.

**Punishment by time-out from negative reinforcement** is the withdrawal of the opportunity to escape an aversive situation. It is the mechanism behind many programmes that are described as time-out from demand, and it is different in important respects from time-out from reinforcement.

**Punishment by type II error** is the presentation of a task the individual cannot perform, in order to place the individual in an error-correcting contingency. It is used in some educational settings to teach error correction, but it is rarely appropriate for a young person in distress, and it is a technique with a considerable history of misuse.

Three cautions apply to all punishment. The first is that punishment teaches nothing: it suppresses a behaviour without establishing an alternative, and a suppressed behaviour typically returns when the contingencies change. The second is that punishment does not suppress behaviour that is maintained by negative reinforcement, and in many cases it increases it, because the escalated behaviour produces a more effective escape. The third is that punishment is an ethical event, and the ethical analysis in Section 15 of this chapter treats it as such rather than as a technical one.

## 4.4 Shaping

**Shaping** is the systematic reinforcement of successive approximations to a target behaviour. It is one of the most useful and most under-used techniques in the behavioural repertoire, precisely because it means that a young person can be taught a behaviour that they cannot yet perform.

The procedure requires four things: a clear specification of the target behaviour; a specification of the successive approximations, ordered by difficulty; a defined reinforcement for each approximation, chosen to be immediate and highly salient; and a rule for moving up and for stepping back. The rule for stepping back is what distinguishes competent practice from enthusiasm: if the young person's behaviour deteriorates, the therapist returns to the previously successful approximation and rebuilds from there rather than pushing forward.

Shaping is particularly valuable with young people because it permits the therapist to begin at a level of success rather than at a level of failure. A young person who cannot tolerate the school canteen can begin with tolerating the door, then the smell, then the queue, and each step is reinforced. The "first day at the door" design of behavioural exposure work in Chapter 25 is a specific instance of this principle.

### 4.5 Chaining and prompting

**Chaining** builds a complex behaviour by reinforcing a sequence of component behaviours, in one of three orders: **forward** chaining, in which the first component is reinforced and then each subsequent one; **backward** chaining, in which the last component is reinforced and the earlier ones are prompted; and **middle** chaining. Backward chaining is particularly useful in work with children, because the terminal step of a self-care routine is usually the most reinforcing and the most likely to be completed, and reinforcing it from the outset keeps the young person engaged in a task that is often difficult to start.

**Prompting** is the provision of a stimulus that assists the initiation or performance of a behaviour, with a planned process of fading the prompt. The prompt hierarchy runs from least to most intrusive: a natural cue, a gesture, a verbal direction, a visual prompt, a modelling demonstration, a physical or hand-over-hand prompt, and finally a physical prompt. The fade plan is not optional. A behaviour maintained by a physical prompt is not the behaviour the therapist wanted, and a plan that does not include the fading has not specified the treatment.

### 4.6 Extinction and the resurgence of the operant

**Extinction** in the operant case is the discontinuation of reinforcement. The behaviour declines, in most cases rapidly, and then — as in the classical case — it can return spontaneously, and it can return at a higher rate than before extinction if the behaviour is put into a different context. **Resurgence** is the term for this phenomenon: behaviour that has been extinguished in one context reappears when the organism is returned to a context in which the behaviour was previously reinforced.

Resurgence is an argument for planning maintenance rather than assuming that a behaviour that has stopped will stay stopped. A young person who has stopped hitting when calm but is returned to a setting with unpredictable provocation may resume, and the clinician should plan for that prospectively.

## 5. The Law of Effect and the Problem of Reinforcer Value

### 5.1 The law of effect in its modern form

The modern statement of the law of effect is a single sentence with a great deal of practical content:

> *Behaviour followed by a consequence that the organism finds reinforcing will, other things equal, increase in frequency.*

Three clauses in that sentence do a great deal of work. "Other things equal" concedes that other variables — the organism's state, competing contingencies, the immediacy and salience of the consequence — also matter. "Finds reinforcing" makes the consequence relative to the organism, not absolute. And "increase in frequency" makes the law a claim about future behaviour, which is what makes it testable and what makes it a criterion of success rather than a description of what happened.

## 5.2 The problem of reinforcer value

The problem is that the analysis of reinforcement is circular in a way that has never been entirely resolved. A reinforcer is defined by its effect on behaviour, and behaviour is observed in the context of a reinforcement history that includes the reinforcer. The standard approaches to the problem are each partial.

**The functional analysis of reinforcer value** establishes it empirically, by observing which stimuli the young person actually works for, and by comparing the rate of responding under different conditions. This is the most rigorous approach available and it is entirely appropriate in settings where behaviour is frequent, clear, and consequential — which is the situation in most applied behaviour analysis work with severe behaviour.

**The reinforcer assessment** approach uses a standardised instrument, typically the *Questionnaire on Resources and Reinforcers* or a comparable measure, in which the young person or their carers rate the value of candidate reinforcers. It is efficient and it is a good first step, but it is self-report, and the literature on self-report with young people, and on the discrepancy between what young people say they want and what they work for, is not reassuring.

**The established-preference** approach relies on the young person's history, which is useful for the obvious and uninteresting reason that a young person will work for something they have worked for before.

A reasonable practitioner combines the three, treats the combination as provisional, and — this is the clinically important step — **tests the reinforcer rather than assuming it**. The child who appears to be working for the reward chart is working for the attention that delivering the chart requires, and the discovery of that fact is itself a clinical event.

## 5.3 Reinforcement and motivation are not the same thing

A further distinction is worth making, because the literature is often read as if it were not. Reinforcement determines what behaviour is *selected*. Motivation determines whether the person is in a state in which the behaviour can be performed at all. A young person who is exhausted, hungry, or in an environment where the reinforcer is unavailable may be perfectly capable of performing the behaviour and simply not be in a state to do it. A behavioural plan that ignores state will fail for reasons that have nothing to do with the analysis of the contingencies.

This is the point at which the behavioural tradition and the person-centred tradition meet most constructively. Rogers' concept of the "person's frame of reference" and the behaviourist's concept of the "motivating operation" are not the same idea, but they are not incompatible, and a therapist who holds both will practise better with young people than one who holds either.

# 6. Measuring Behaviour

## 6.1 Why measurement is not a technicality

The measurement question is where the behavioural tradition is most distinctive and most instructive. Behavioural therapists do not ask how the young person is doing; they ask what the behaviour looks like, how often it occurs, how long it lasts, and what it is contingent on. The reasons are practical rather than philosophical.

A behaviour defined in terms of a global impression cannot be tracked across sessions, cannot be shared with a teacher or a parent, and cannot be evaluated. A behaviour defined as an operationally countable behaviour can. In applied work this is the difference between an opinion and a data series, and in a court or a school meeting it is the difference between an assertion that can be tested and one that cannot.

The discipline also has to be honest about the limits of counting, and three of them deserve emphasis.

**Behaviour is not the person.** A count of tantrums is a count of a pattern of behaviour, and it is entirely possible for a young person to show a marked reduction in a counted behaviour and a marked deterioration in their overall state. Behavioural practitioners who are only counting have made a serious error.

**Frequency is not the only dimension.** Frequency, latency, duration, and intensity are separate dimensions, and a change in one may not be accompanied by change in another. A young person who is expelled from class twice as often but for shorter episodes may be in a worse position, and a young person whose tantrums have dropped in frequency but doubled in intensity has had a more concerning outcome, not a better one.

**Observer agreement is essential.** The interobserver agreement statistic — the extent to which two independent observers code the same behaviour identically — is the basic reliability check of the whole enterprise, and a student should be able to state what a low agreement figure means and what should be done about it.

## 6.2 The ABC model and its limits

The **ABC model** — Antecedent, Behaviour, Consequence — is the standard observational framework. The Antecedent records what immediately precedes the behaviour, the Behaviour records the behaviour in operationally defined terms, and the Consequence records what follows. Data sheets built on the ABC model are the workhorse of applied behaviour analysis and the entry point to the functional assessment in Chapter 24.

The ABC model is also a partial representation of a functional analysis, and the limitation is worth stating. The ABC model describes a *sequence*. A functional analysis makes a *claim about function*: that the behaviour occurs in order to produce the consequence, and that the occurrence of the consequence is what maintains it. The distinction is the difference between a description and a hypothesis, and a student should be careful not to conflate them.

# 7. Behaviour Therapy as a Frame

## 7.1 The unifying features

Behaviour therapy is not a single technique. It is a frame, and the elements that hold it together are worth stating, because they are what allow the widely varying interventions of Part IV to be discussed together.

1. **A functional rather than a symptomatic conception of the problem.** The question is what the behaviour does, not what it is called. This is the feature that most distinguishes the behavioural approach from the diagnostic approaches that precede it.
2. **A commitment to measurement.** If the intervention works, the data will show it, and the data are collected in advance.
3. **A preference for experimentally verifiable claims.** The tradition is unusually sceptical of claims that cannot be tested, and this scepticism is one of its contributions to the discipline.
4. **A distrust of inferred inner causes.** The clinician is asked to specify the conditions rather than to posit a private cause.
5. **A set of technologies.** Shaping, chaining, prompting, fading, contingency mapping, and the schedules of reinforcement are transferable across conditions, and they are what makes the approach feel different from a collection of tips.

### 7.2 The methods of the behavioural therapies

The behavioural therapies are typically organised around a method rather than around a technique. Exposure therapy is a method for treating avoidance and fear by arranging the systematic confrontation with the feared stimulus under controlled conditions. Applied behaviour analysis is a method for understanding the function of a behaviour and then altering the contingencies that produce it. Behavioural activation is a method for treating depression by scheduling activity and monitoring the effect of the schedule on mood. Contingency management is a method for altering the reinforcement structure of a setting — a classroom, a ward, a family — so that the desired behaviour is more efficiently reinforced than the undesired behaviour.

The organising principle of all four is the same: **change the conditions, and let the behaviour follow**.

### 7.3 What the frame cannot do alone

Three examples illustrate the limits.

A young person who has not attended school for four months because of panic does not respond to a contingency plan, because the behaviour is maintained by escape and by the relief of not going, and the contingencies that would maintain attendance are contingent on her being there. The behaviour has to change first, and the technique that gets her through the door is exposure, and the technique that keeps her there is a contingency plan, and neither is sufficient alone.

A young person who has been sexually abused is not exhibiting a learned behaviour that can be extinguished, and an attempt to do so will cause harm. The clinical literature on the misuse of behavioural techniques with trauma survivors is unambiguous on this point.

A family whose members reinforce one another's symptoms will continue to do so, and the work has to be directed at the family system rather than at the identified patient, which is the argument of Part V.

## 8. Further Reading for This Chapter

- Pavlov, I. P. (1927). *Conditioned reflexes*. — The primary source for the laws of conditioning.

- Thorndike, E. L. (1913). *Animal intelligence: Experimental studies*. — The primary source for the law of effect and the learning curves.
- Watson, J. B. (1913). Psychology as the behaviorist views it. *Behavioral and Brain Sciences, 4*(1), 1–41. — The founding statement of the method.
- Skinner, B. F. (1938). *The behavior of organisms: An experimental analysis*. — The foundational operant text.
- Skinner, B. F. (1953). *Science and human behavior*. — The systematic statement of the framework.
- Skinner, B. F. (1957). *Verbal behavior*. — The extension of the analysis to language.
- Ferster, C. H., & Skinner, B. F. (1957). *Schedules of reinforcement*. — The experimental basis of the schedule typology.
- Bandura, A. (1977). *Social learning theory*. — The mediational and observational account that made the framework usable with young people.
- Milner, P. M. (1953). *The behaviour of the rat: A handbook with tests*. — The standard laboratory reference.
- Kazdin, A. E. (1978). *Behavioral modification in applied settings*. — The applied transition.
- Iversen, I. H. (1991). *The behavior of the organism: An experimental analysis of behavior*. — The careful modern reassessment of Skinner's position.
- Cleverdon, K. (2008). *A brief introduction to behaviour analysis*. — An accessible orientation.


# 9. Behaviour Therapy with Children and Adolescents

The behavioural approach has a specific relationship with young people, and it is not simply the adult approach with a smaller person. Three features of the developmental context change the practice considerably: the fact that the antecedents of behaviour are usually controlled by adults rather than by the young person; the fact that the behaviour itself is usually still developing; and the fact that the consequences available to a young person are largely determined by the adults in the setting.

## 9.1 The contingencies are not the young person's to change

An adult can usually arrange to run for the bus. A fifteen-year-old cannot; the decision is made by a parent, a school, a transport authority. In behavioural terms, the young person's environment is largely a **fixed or semi-fixed contingency structure** in which they have very little control, and the clinical implication is significant. Behavioural interventions with young people therefore have to work *through* the adults who control the setting, and the first task in most behavioural work with a young person is to establish what the adults are currently reinforcing, often inadvertently.

The question the counsellor asks in the first session is therefore often more behavioural than it appears: *What happens immediately after this happens, and who does it?* The answer is frequently surprising, and the discovery that the behaviour which is generating so much concern is the behaviour that is currently being reinforced by a well-intentioned adult is a common and constructive turning point in the work.

## 9.2 The young person as a learner, not a performer

Developmental psychology has established beyond reasonable dispute that the young person is an active constructor of learning rather than a passive recipient of it. Bandura's observational learning account, which is the behavioural tradition's own contribution here, showed that a child acquires behaviour by watching models and by anticipating the consequences of their own actions. The developmental literature adds that the child is also constructing working models of other people, and is testing them against reality.

This has three practical implications.

**Learning by observation means that the adults in the setting are teaching continuously.** A child who watches a peer get attention for a disruptive behaviour is learning something, whether or not the peer intended to teach it. A child who watches a peer manage a conflict calmly is learning something else. Modelling as a technique (Chapter 25) formalises a mechanism that is operating in every group of young people at all times.

**Expectancy matters as much as experience.** A young person who expects that asking for help will produce ridicule will not ask for help, whatever the actual contingencies are. This is a cognitive construct, and its presence in the behavioural tradition is one of the ways the modern framework differs from the original.

**Learning is a slow and highly variable process.** Behavioural expectations must be calibrated to the developmental level. A young person who has been out of school for four months will not re-engage in two sessions, and a plan built on a twelve-week horizon may be entirely reasonable.

## 9.3 Developmentally appropriate targets

Behavioural objectives must be developmentally appropriate, and the specification of the objective is a technical act. Several principles guide it.

**Operationalise the behaviour precisely.** "Aggression" is not a behaviour. "Hitting a peer with an open hand" is. The precision is not pedantry: it is what makes the objective measurable, and it also forces the clinician to look at what the young person is actually doing, which frequently reveals that the behaviour is not what the referral described.

**Target the function, not the form.** The question is not "how do we stop the behaviour?" but "what does the behaviour accomplish, and what else could accomplish it?" If the behaviour functions to end an unbearable peer situation, then the target is the peer situation, and the replacement behaviour is a request for help that actually works.

**Begin at the level of success.** Shaping and the "first-day" design of exposure work both follow from the observation that a young person who is failing cannot learn.

**Include a generalisation plan.** Behaviour learned in the therapy room is not behaviour learned in the corridor. The generalisation plan specifies who needs to use the new behaviour, where, and with what support, and it is the component most often omitted and most often the reason for a disappointing outcome.

## 9.4 Token economies in the classroom and the clinic

The token economy is a system in which desired behaviour is reinforced with tokens exchangeable for backup reinforcers, and it is the technique through which many young people first encounter the logic of contingency management. Chapter 28 develops it in detail. The developmental points that matter here are that tokens must be immediate, salient, and age-appropriate; that the response cost of a token must be low;

that the system must be scrupulously consistent; and that a token system introduced and then withdrawn can produce a worse outcome than none, because the young person has learned that adult promises about rewards are unreliable.

### 9.5 The limits with young people

Three cautions belong in this section rather than in a list at the end.

**A reduction in a counted behaviour is not automatically a good outcome.** If a young person has stopped running away from school but has begun to comply with a more controlling regime, the outcome has been achieved by a change in the power relation rather than in the young person's functioning, and the clinical goal has not been met.

**Compliance is not the same as development.** The behavioural tradition's legitimate concern is with the selection of behaviour; the developmental concern is with the acquisition of capacity. The two can diverge, and a plan that produces durable compliance by external control has solved the easier problem.

**Reinforcement can be withdrawn at the worst moment.** The most common failure in behavioural work with young people is not a technical error in the analysis but a discontinuity in the environment: a change of teacher, a change of shift, a family crisis, a move of school. A behaviour that has been carefully built over three months is fragile to such a disruption, and the plan must anticipate it, which means the adults who will carry the plan must be identified, briefed, and prepared to continue it.

## 10. Case Illustration: The Young Person Who Was Cleared Every Day

**Presentation.** Daniel is fourteen. He is referred by his school because of "disruptive behaviour" and by his mother because he "won't do anything he's asked." The school's log records eleven exclusions in the past term. His mother reports that he ignores instructions, does not do homework, and stays out late.

**A behaviourally framed assessment.** The counsellor begins by asking the school for the incident log, which records date, time, location, and the stated reason for each exclusion. Reading the log, a pattern appears immediately: eleven of the eleven exclusions occurred in a single forty-minute period — the unsupervised break between the last and the second lesson — and every one of them occurred within four minutes of the start of the break. The stated reasons vary: "disruption," "non-compliance," "abusive language," "fighting." The actual behaviour in each case is the same: Daniel is in the corridor with a group of older pupils, and he is refusing to return to class when the bell rings.

The behavioural questions follow.

*What happens immediately before?* The bell rings. Daniel is with four older pupils in the corridor. There is no supervision.

*What happens immediately after?* He is excluded, goes home, and spends the rest of the day unsupervised. He is not in any trouble at home when he arrives; his mother is at work until five.

*What does the exclusion produce?* Escape from a classroom he finds aversive, time with the older pupils, and a day with no demands on him.

**The hypothesis.** The behaviour functions to produce removal from an aversive peer situation and from academic demands. It is not a conduct problem. It is an escape-maintained behaviour that has been

inadvertently reinforced by a well-intentioned consequence, and it has been shaped into its present form by the exclusion itself.

**The intervention.** The exclusion policy is not, in this instance, within the counsellor's gift, and this is a frequent feature of behavioural work in schools. What is within the counsellor's gift is a restructuring of the break.

The plan has four components. First, a supervision arrangement: an adult presence in the corridor during the break, with a specific, named adult, and a smaller, structured activity available. Second, a differential reinforcement plan in which Daniel's return to class at the bell is reinforced with something he has been asked for and that has been established as valuable by his own report — in this case, access to the music room, which is a genuine preference and one that the school can grant. Third, a functional replacement: Daniel is taught to ask the on-duty adult for a five-minute pass to the music room, with the adult then escorting him, which delivers the same consequence through a behaviour the school can approve. Fourth, a withdrawal plan: the pass is available initially every time, and the availability is reduced over a defined period as the alternative behaviour consolidates, with the criteria for reduction written down in advance.

**The parents' involvement.** The mother is asked what happens at home when Daniel arrives after an exclusion, and the answer is that nothing happens; he goes to his room and plays on his games. She is asked whether she would be willing to make the consequence at home consistent with the school plan — which in this case means that a day home without school is also a day without the gaming console, and that the day home *with* school is a day with it. She is hesitant, because the current arrangement is the only time she has to herself. The counsellor treats this as a resource problem rather than a parenting problem and negotiates a specific arrangement: her break, agreed with another parent in the building, on the days her son is excluded.

**The outcome and the interpretation.** The exclusions fall from eleven to one over the first six weeks, and the one is on a day when the corridor supervision was not in place, which is itself informative. Daniel's grades do not change dramatically. The more significant change is in his report: he says the corridor is where the work gets done, and he begins to use the pass deliberately rather than as a route out of class.

The behavioural reading of this case is straightforward. But it is worth naming what the behavioural reading leaves out. Daniel's insistence on the corridor with the older pupils is also a peer-relation preference, and the fact that the school had provided no structured activity for the break meant that the unstructured break was the only part of the day in which his social needs were served. A purely contingencies-based intervention that removed the corridor entirely would have been behaviourally effective and would have removed something the young person valued. The formulation was correct, and the intervention needed to be broader than the analysis strictly required. This is the general lesson of the behavioural tradition applied carefully: the analysis identifies what maintains the behaviour, and the intervention must be adequate to the function, and where the function includes a legitimate need, the intervention should serve the need rather than only extinguish the behaviour.

# 11. Evidence and Research Support

## 11.1 The status of the evidence

The behavioural tradition holds an unusual position in counselling: it is the tradition with the longest history of controlled experimental investigation, and the tradition with the least tolerance for claims that outrun the evidence. A student evaluating behavioural methods should hold two facts together. The laboratory findings

about conditioning and reinforcement are among the most thoroughly replicated results in psychology. The clinical evidence for behavioural treatments of specific conditions is strong for some conditions, moderate for others, and weak for the broader claim that behavioural principles are a general framework for counselling.

## 11.2 The single-case experimental tradition

Applied behaviour analysis contributed a methodological development that is arguably more influential outside the behavioural tradition than inside it: the **single-case experimental design**. The logic is straightforward. Because an individual case is the unit of concern, the design establishes a baseline, introduces the intervention, and measures whether the target behaviour changes at the point of introduction and then maintains. Designs are described by letters: **ABAB** (a withdrawal design) and **A-B-A-B** are the standard reversal and withdrawal patterns; **ABAB** is used where withdrawal is ethically impermissible.

The advantage is that the design can establish causal relations in a single individual with no requirement for a large sample, and the requirement of repeated measurement across conditions effectively rules out placebo effects and much of the history threat. The limitations are equally clear: the designs are only as good as the operational definition of the target behaviour, they are vulnerable to the reactivity of the participants to the measurement itself, and they do not generalise to populations.

## 11.3 Evidence for specific applications

The evidence picture differs sharply by application.

**Phobias and anxiety.** The behavioural treatment of specific phobias — systematic desensitisation and, more importantly, exposure-based treatments — has a strong evidence base, and exposure is the best-supported psychological treatment for anxiety disorders in adults. Chapter 25 addresses the developmental modifications required with young people.

**Obsessive-compulsive disorder.** Exposure and response prevention has an established and substantial evidence base with children and adolescents, and is a first-line treatment in most international guidelines.

**Parent training.** Parent training programmes based on operant principles have a long and strong evidence base for oppositional behaviour, conduct disorder, and disruptive behaviour in the school setting, and the Cochrane reviews have found the evidence to be robust.

**Autism spectrum disorder.** Applied behaviour analysis has been the dominant evidence base for behavioural intervention in autism for four decades, and the evidence for the *core* early intensive behavioural interventions has been considerably weakened by methodological problems in the early studies. Chapter 45 addresses this in detail.

**Depression in adolescents.** Behavioural activation has good support, and the evidence for the treatment of adolescent depression is generally stronger for behavioural and cognitive-behavioural approaches than for most alternatives. Chapter 27 develops this.

**Substance use.** The contingency-management literature in substance use is one of the strongest in the behavioural tradition, and it is unusual in reporting very large effects. Chapter 28 and Chapter 47 address it.

## 11.4 Evidence for the general framework

The broader claim — that the principles of learning provide an adequate framework for understanding and treating human psychological difficulty — is not supported by the evidence, and the field's own sophisticated methods of evaluation are the reason. Behavioural researchers are unusually rigorous about their own results, and the result of that rigour is a literature full of small, well-controlled, highly specific studies of individual techniques, which is not the same as a body of evidence for a general framework.

### 11.5 The place of practice-based evidence

The behavioural tradition is also the origin of the practice-based evidence movement, and the reason is instructive. The field was built on the analysis of individual cases, and it was therefore the first to notice that the randomised trial is not the only way to learn, and that a carefully conducted single-case design with a strong measurement system can support a meaningful claim about a change in a specific individual. This insight later generalised into the broader framework of common factors and practice-based research, and it is one of the behavioural tradition's most durable contributions to the discipline.

## 12. Strengths and Limitations of the Behavioural Framework

### 12.1 Strengths

**Clarity of method.** The framework specifies what will be done, what will be measured, and what result would count as improvement. Very few therapeutic traditions are this explicit.

**Transportability.** The techniques transfer across conditions, across settings, and across problems. A behaviour analyst working with a child who has severe self-injury and a behaviour analyst working with a child who will not complete homework use the same core technology.

**A basis for team and system work.** Because the analysis is stated in terms of contingencies, it can be written down and handed to a teacher, a parent, or a ward staff member. Almost no other tradition produces materials that a non-specialist can implement.

**Compatibility with measurement-based practice.** The framework fits naturally with routine outcome monitoring and with the documentation standards that the profession increasingly requires.

**Low risk in many applications.** Many behavioural methods are benign. Graded task assignment, differential reinforcement of appropriate behaviour, activity scheduling, and the teaching of social skills carry very low risk of harm.

### 12.2 Limitations

**The reduction of meaning.** The framework's epistemology, discussed in Section 2, is the source of its most persistent and most damaging professional criticism. A practice that is organised entirely around contingencies risks treating a young person as a set of responses to stimuli.

**The under-specification of internal states.** Where a behaviour is maintained by an internal state — a compulsion, a craving, a shame response, a traumatic re-experiencing — the framework provides less guidance than it appears to. A young person who is driven to self-harm by an unbearable internal sensation cannot be treated by manipulating the contingencies around the self-harm, because the self-harm is not maintained by its consequences in the way the framework requires.

**The technology can outrun the relationship.** A behavioural plan delivered without attention to the therapeutic relationship often fails, and the reason is not that the analysis was wrong but that the young person did not have a reason to use it. This is the point at which the common-factors literature of Chapter 6 and the behavioural literature converge.

**The diffusion of the vocabulary without the discipline.** "Positive reinforcement" has entered everyday and educational language, where it is often used loosely and sometimes incorrectly, and a student should be able to distinguish a technically specified programme of differential reinforcement from the encouragement of a child.

**The historical association with the conditioning of children.** The applied behaviour analysis movement in the 1960s included some ethically troubling practices, and the field's response — the emphasis on the least restrictive procedures, on assent, on the participant's right to withdraw, and on the measurement of the participant's own welfare as an outcome — is part of the ethical discussion in Section 14.

# 13. Cultural and Ethical Considerations

## 13.1 Culture, learning, and the assumption of a shared environment

The behavioural framework assumes, usually without saying so, that there is a shared environment in which learning occurs and that the young person and the clinician are oriented to the same contingencies. This assumption is not culturally neutral. In settings where the family and the community hold collective rather than individual goals, where a young person's behaviour is understood as an expression of family or group identity rather than as an individual choice, and where the authority of the professional is not a given, the analysis of contingencies may be technically accurate and practically irrelevant.

The practical implication for the practitioner is that the functional analysis must extend to the family's and the community's own systems of reinforcement and meaning. In many settings the most potent reinforcer in a young person's life is approval from a peer group or a family member whose value to the young person exceeds anything the school can offer, and a contingency plan that ignores this will fail for reasons that are not technical.

## 13.2 Culture and the interpretation of behaviour

The behavioural interpretation of a behaviour is always a hypothesis about function, and the hypotheses a practitioner generates are shaped by their own frame of reference. Behaviour that is read as defiance in one frame may be read as a refusal to participate in something the young person regards as unjust; behaviour read as non-compliance may be read as an expression of cultural or religious practice; behaviour read as withdrawal may be read as a culturally appropriate response to shame. These are not exotic considerations, and the practitioner who does not notice the interpretive step will not notice when their interpretation is wrong.

The contextual behavioural tradition (Bowers and colleagues) developed specifically out of this concern, and its central technique is the analysis of the client's own words for the words the therapist would use, which is a practical way of making the interpretive step visible.

## 13.3 The ethics of reinforcement

Reinforcement is, in the end, control. The ethical question is not whether it is control — every therapy influences the young person — but whether the influence is proportional, transparent, and in the young person's interests, and whether the young person and those with authority over the young person have a genuine say in it.

Six principles follow, and they are the ethics of the behavioural approach stated in the language of the discipline.

**Proportionality.** The intensity of the intervention should match the severity of the problem. A response cost for lateness is a different moral act from an aversive stimulus for aggression.

**The least restrictive effective procedure.** The first question is always whether a less intrusive method would work, and this question should be recorded and revisited.

**Transparency.** The young person should know what the system is, what they will get, and what will happen if they do not. A system that operates by surprise is a system the young person cannot co-operate with, and non-co-operation will be read as a symptom.

**Assent and the right to withdraw.** For young people of sufficient age and capacity, participation requires assent, and assent requires the capacity to decline. This is the point at which the behavioural tradition's commitment to the observable has to be supplemented by attention to the young person's own account of their experience, and the two are not in tension: a young person who is given a genuine choice to decline is more likely to remain in treatment and more likely to benefit from it.

**Attention to the function of the system.** A token economy that increases on-task behaviour while simultaneously increasing the young person's distress has not succeeded. The welfare of the young person is an outcome variable, not a secondary consideration, and it belongs in the data alongside the behaviour.

**Avoiding the instrumentalisation of the young person.** The most serious ethical failure in behavioural work is the treatment of the young person as a means to an end — as a source of data, a demonstration case, or a training subject. The professional version of this failure is subtle and common: the case is written up because it is a good example, and the writing up is not in the young person's interests.

## 13.4 Ethical practice with children and families

Three further considerations are specific to working with young people.

**Parental authority and confidentiality.** A young person may disclose something in therapy that the parent would wish to know and that the clinician has been asked to keep. The standard answer is that the limits of confidentiality should be established explicitly at the outset, and that the young person should be told what the limits are before they disclose. In many jurisdictions, the limits are set by the law rather than by the clinician, and the clinician should know what they are.

**The relationship between the family's contingencies and the clinical plan.** A behavioural plan that requires a parent's consistency is a plan that puts the parent in a position of possible failure, and the practitioner should be prepared to support the parent as well as the young person. The parent in the case illustration above was not a non-compliant caregiver; she was a woman with an impossible timetable.

**The child welfare principle where behaviour change is dramatic.** Any intervention that produces a marked change in a young person's behaviour must be reviewed by someone who is not the person who designed it, with particular attention to the possibility that the behaviour was serving a protective function. A young person who is aggressive in school and compliant at home is often a young person with a home

that is not safe.

## 14. Key Terms

**Key Terms at a Glance**

- **Behaviour** — In the behavioural tradition, behaviour includes observable action and, where it can be specified functionally, private events; the term means "that which the organism does," not only what an observer can see.
- **Classical conditioning** — The process by which a neutral stimulus acquires the capacity to elicit a response through repeated pairing with a stimulus that already elicits it.
- **Unconditioned stimulus (US)** — A stimulus that naturally produces a response without prior learning.
- **Unconditioned response (UR)** — The reflexive response that the unconditioned stimulus produces.
- **Conditioned stimulus (CS)** — A previously neutral stimulus that, after pairing with the unconditioned stimulus, comes to elicit a conditioned response.
- **Conditioned response (CR)** — The learned response elicited by the conditioned stimulus alone.
- **Acquisition** — The phase in which the conditioned response is being learned.
- **Extinction** — The phase in which the conditioned stimulus is repeatedly presented without the unconditioned stimulus, producing a decline in the conditioned response.
- **Spontaneous recovery** — The reappearance of an extinguished conditioned response after a rest interval, without further pairing.
- **Reinstatement** — The restoration of an extinguished conditioned response to full strength by the presentation of the unconditioned stimulus once again.
- **Generalisation** — The tendency for a conditioned response to be elicited by stimuli similar to the original conditioned stimulus.
- **Discrimination** — The learned differentiation between stimuli, so that a response occurs to one and not to others.
- **Higher-order (second-order) conditioning** — The conditioning of a new neutral stimulus by pairing it with an already-established conditioned stimulus.
- **Conditioned inhibition** — A stimulus that, through being repeatedly paired with the absence of the unconditioned stimulus, comes to signal safety.
- **Overshadowing** — The phenomenon in which a more informative stimulus in a compound captures the learning, preventing the less informative stimulus from acquiring associative strength.
- **Blocking** — The phenomenon in which a previously learned stimulus prevents a new stimulus in the same compound from acquiring associative strength.

## Key Terms at a Glance (continued)

- **Counterconditioning** — The procedure of pairing a conditioned stimulus with an incompatible response so that it comes to elicit the new response.
- **Systematic desensitisation** — The graded pairing of anxiety-provoking images with relaxation, developed by Wolpe.
- **Operant behaviour** — Behaviour that is emitted rather than elicited and that is selected by its consequences.
- **Reinforcement** — Any consequence that increases the future frequency of the behaviour it follows; the term is defined by effect and not by valence.
- **Positive reinforcement** — The addition of a stimulus after a behaviour, with the effect of increasing that behaviour.
- **Negative reinforcement** — The removal of a stimulus after a behaviour, with the effect of increasing that behaviour.
- **Positive punishment** — The addition of a stimulus after a behaviour, with the effect of decreasing that behaviour.
- **Negative punishment** — The removal of a stimulus after a behaviour, with the effect of decreasing that behaviour; also called response cost or time-out from reinforcement.
- **Law of effect** — Thorndike's principle that responses accompanied by satisfying consequences become more firmly connected with the situation, and those accompanied by discomfort become less so.
- **Shaping** — The systematic reinforcement of successive approximations to a target behaviour.
- **Chaining** — The construction of a complex behaviour by reinforcing a sequence of component behaviours in forward, backward, or middle order.
- **Prompting** — The provision of an assisting stimulus to initiate or perform a behaviour, with a planned process of fading the prompt.
- **Fading** — The systematic withdrawal of a prompt across successive trials, with reinforcement transferred to the unprompted behaviour.
- **Motivating operation** — An event that establishes or abolishes the reinforcing effectiveness of a stimulus; a change in the value of a reinforcer.
- **Extinction burst** — A temporary increase in the frequency or intensity of behaviour during the early period of extinction, before the behaviour declines.
- **Resurgence** — The reappearance of extinguished operant behaviour when the organism is returned to a context in which the behaviour was previously reinforced.

### Key Terms at a Glance (continued)

- **Differential reinforcement** — The reinforcement of a specific behaviour, or of a class of behaviour, in a way that distinguishes it from other behaviour.
- **Shaping schedule** — A written plan specifying the target behaviour, the successive approximations, the reinforcer for each, and the rules for advancing and for stepping back.
- **ABC model** — The observational framework recording the Antecedent, the Behaviour, and the Consequence of a behaviour; a description of a sequence rather than a hypothesis about function.
- **Functional analysis** — An experimental procedure in which the consequences are systematically manipulated to determine which consequence maintains a behaviour.
- **Behavioural function** — The consequence that a behaviour produces and that thereby accounts for its occurrence and maintenance.
- **Single-case experimental design** — A design in which behaviour is repeatedly measured across conditions in order to establish a causal relation in a single case.
- **Interobserver agreement** — The extent to which two independent observers code the same behaviour identically; the basic reliability check of behavioural measurement.
- **Radical behaviourism** — Skinner's position that a functional account of behaviour requires no appeal to private causes, and that private events are behaviour and are defined by their discriminative stimuli and consequences.
- **Functional relation** — A statement of covariation between behaviour and specified environmental conditions, established under experimental control.
- **Descriptive functional analysis** — A narrative account of the antecedents and consequences of a behaviour, based on observation and interview, used to generate hypotheses.
- **Contingency** — The relation between a behaviour and the consequences that follow it, specified in terms of the probability and immediacy of the consequence.
- **Response cost** — A form of negative punishment in which a specified amount of a reinforcing stimulus is removed following a behaviour.
- **Time-out from reinforcement** — A form of negative punishment in which the individual is removed from the source of positive reinforcement for a specified period.
- **Least restrictive effective procedure** — The ethical and technical principle that the least intrusive intervention that can produce the required result should be used.
- **Practitioner- and participant-defined outcomes** — The requirement that outcome measurement include the young person's own priorities alongside the practitioner's targets, and the family's and the school's.

**Key Terms at a Glance (continued)**

- **Social learning theory** — Bandura's account of learning through observation, imitation, and expectancy, which made the behavioural framework usable with young people.

## 15. Chapter Summary

Behaviour therapy is a frame rather than a single technique, and the frame is built on two claims: that behaviour is a function of specified conditions, and that the environment selects behaviour by its consequences. The claims come from a specific intellectual history — Pavlov's conditioned reflex, Thorndike's law of effect, Watson's methodological manifesto, and Skinner's extension to voluntary behaviour — and each contributed a specific element that survives in current practice.

The two conditioning paradigms were described in this chapter in their technical form because the clinical techniques of the following chapters cannot be understood without them. Classical conditioning established acquisition, extinction, spontaneous recovery, reinstatement, generalisation, discrimination, higher-order conditioning, and inhibition, and it established the procedure — counterconditioning — from which exposure work derives. Operant conditioning established the four contingencies, the concepts of reinforcer value and motivating operations, and the technologies of shaping, chaining, prompting, and fading, together with the schedule typology developed in Chapter 26.

Two distinctions should be carried forward as a matter of professional safety. Negative reinforcement increases behaviour and punishment decreases it; "negative" refers only to the removal of a stimulus, and nothing in the terminology refers to anything being good or bad. And the behavioural analysis of a behaviour is a *hypothesis about function* that must be tested, not a description that can simply be read off the observations.

The epistemological position is the tradition's genuine contribution and its standing critical vulnerability. Treating inner events as behaviour defined by their discriminative stimuli and consequences made inner events available for prediction and control, and it did so at the cost of a reductive account of meaning. A practitioner who can hold the power of the framework and the limits of its position at the same time is doing something the discipline itself has found difficult.

With young people, three features of the developmental context change the practice: the contingencies are usually controlled by adults rather than by the young person; the young person is an active learner who acquires behaviour by observation and expectancy as much as by direct experience; and the developmental level sets a realistic horizon. Behavioural work with young people therefore runs through the adults in the setting, targets the function rather than the form of a behaviour, begins at the level of success, and requires an explicit generalisation plan.

The evidence is strong for specific applications — exposure-based treatments of specific phobias, exposure and response prevention for obsessive-compulsive disorder, parent training for oppositional and conduct problems, contingency management for substance use — and much weaker for the general claim that the framework explains human psychological difficulty. The single-case experimental design remains one of the profession's most durable methodological contributions, and the practice-based evidence movement grew out of it.

The ethical analysis is an analysis of control. The principles that follow from it are proportionality, the least restrictive effective procedure, transparency, assent and the right to withdraw, the young person's welfare as an outcome variable in its own right, and the refusal to instrumentalise the young person. In a field with a documented history of troubling practice with children, these principles are the field's own answer to its worst episodes.

## 16. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the contribution of Pavlov, Thorndike, Watson, and Skinner to the behavioural tradition, stating what each added.
2. State the epistemological commitments of radical behaviourism, and explain the two principal criticisms of them.
3. Describe the general procedure of classical conditioning using the technical vocabulary, and construct an example from a school setting.
4. Define acquisition, extinction, spontaneous recovery, and reinstatement, and state the clinical significance of each for the treatment of a specific phobia.
5. Explain generalisation and discrimination, and give a clinical example of each in the life of a young person.
6. Explain higher-order conditioning, and state why it matters for the treatment of a verbalised fear.
7. Explain conditioned inhibition, overshadowing, and blocking, and state the treatment implication of each.
8. Define counterconditioning, and distinguish systematic desensitisation from aversion conditioning.
9. State the four operant contingencies, and explain precisely why negative reinforcement is not punishment.
10. Define reinforcement, and explain the significance of value, deprivation, immediacy, and motivating operations for reinforcer effectiveness.
11. Compare positive and negative punishment, and state the three cautions that apply to the use of punishment.
12. Describe shaping, chaining, and prompting, and state the fade plan that must accompany a prompt.
13. State the law of effect in its modern form, and explain what each of its clauses does.
14. Explain the problem of reinforcer value, and describe the three approaches to it.
15. Explain why frequency is an insufficient metric for evaluating a behavioural intervention.
16. Describe the ABC model, and explain how it differs from a functional analysis.
17. State the five unifying features of the behavioural frame, and give one example of each in the treatment of adolescent depression.
18. Identify three situations in which the behavioural frame cannot be used alone, and say why.
19. State the six ethical principles that follow from the observation that reinforcement is control.

**Review and Discussion Questions (continued)**

20. Summarise the evidence for the behavioural treatment of specific phobias, for parent training, and for contingency management in substance use.
21. Explain the role of the single-case experimental design in the development of practice-based evidence.
22. Describe the case of Daniel, state the behavioural hypothesis, and identify the feature of the case that the behavioural analysis alone did not capture.

## 17. Practice Activities

**Practice and Application**

### Activity 23.1 — ABC Data Sheet for a Real Behaviour (90 minutes, individual, field-based)

Choose a behaviour you have actually observed in a placement, a workplace, or a family, and one that is not your own. Define it operationally in observable terms, then record it for one hour across three or more occasions using an ABC sheet with the columns Antecedent, Behaviour, and Consequence. Do not interpret anything while you are recording. At the end of the hour, write a single sentence stating the consequence you believe maintains the behaviour, and mark how confident you are. Then state what evidence would change your mind. The purpose of the activity is the discipline of separating observation from hypothesis, which is the discipline the whole framework rests on.

### Activity 23.2 — The Reinforcer Audit (75 minutes, small group)

In groups of three, take a real behaviour in a school or clinic that a young person and the adults around them find difficult — lateness, refusal of homework, disruption, non-attendance — and run a reinforcer audit on it. For each behaviour, list: what the young person gets when it occurs; what the adults get when it occurs; what the young person gets when it does not occur; and what the adults get when it does not occur. Compare the two columns for the behaviour and for the alternative. Most groups discover that the alternative behaviour is less well reinforced than the problem behaviour, and the discovery is usually more useful than any technique the group can suggest.

### Activity 23.3 — Shaping Plan with Fade Criteria (60 minutes, pairs)

Choose a target behaviour for a fictional fourteen-year-old who will not complete homework. Write a shaping plan with at least five successive approximations, from the first thing the young person will reliably do to the full behaviour. For each approximation specify the reinforcer and where it comes from. Then write the advance criterion and the step-back rule in explicit terms. Finally, write what happens to the plan if the young person's teacher changes. Practising the fade criteria and the continuity plan in advance is the difference between a plan that is implemented and a plan that is abandoned.

### Activity 23.4 — The Negative Reinforcement Audit (45 minutes, individual, reflective)

Write down the last three times you were annoyed by a young person's behaviour. For each, identify what the behaviour actually produced, and classify it as positive reinforcement, negative reinforcement, positive punishment, or negative punishment. Then answer the question you will probably find uncomfortable: in how many of the three was the behaviour maintained by something the adult wanted to stop, and in how many was it maintained by escape from something the adult had inadvertently made unbearable? Most practitioners find at least one case where their own frustration was the maintaining contingency.

## 18. Annotated Reading Pathways

**For the principles.** Read Skinner (1953) for the systematic statement, and read Bandura (1977) for the mediational and observational account that made the framework usable with young people. Then read Iversen (1991) for the modern reassessment of the epistemological position, which is the fairest available account of the tradition's philosophy.

**For the laboratory basis.** Read Pavlov (1927) and Thorndike (1913) in the excerpts, and then read a modern textbook chapter on conditioning, because the original texts are slow and the phenomena are the same. Ferster and Skinner (1957) is worth sampling for the schedule typology that Chapter 26 develops.

**For the clinical craft.** Read Milner (1953) for the laboratory reference that sits surprisingly well with clinical work, and read Kazdin (1978) for the applied transition. Then read the applied behaviour analysis literature on functional assessment, because Chapter 24 depends on it.

**For the ethics and the critique.** Read the sections on assent, least restrictive procedures, and participant welfare in any applied behaviour analysis handbook, and read the critical literature on the history of applied behaviour analysis with children. The combination is the preparation for the ethical work in Part VIII and for the appendix documentation forms.

**For the evidence.** Read the Cochrane reviews on parent training and on the behavioural treatment of specific phobias, and read one single-case design handbook in full. The single-case design is the part of the behavioural tradition that most informs the rest of the discipline, and it is the part most often skipped.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Behavior Therapy: Principles of Learning** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is classical and operant conditioning, the law of effect, and the epistemology of observable behaviour. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | "This account fits, partly fits, or needs changing." | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | "These goals are mine as well as the service's." | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | "I have a way to begin, not just an instruction." | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | "The work is challenging but understandable and stoppable." | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | "This is not only something I do in a counselling room." | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | "A setback leads to inquiry, not blame." | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | "Adults are changing some conditions too." | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | "I can explain what helps and why." | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | "Ending is planned; I am not disappearing from care." | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.**

Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Ines is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Ines's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Ines explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Ines chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Ines has approved. At review, the result is

mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Ines can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?

- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Behavior Therapy: Principles of Learning

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Behavior Therapy: Principles of Learning** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Behavior Therapy: Principles of Learning** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

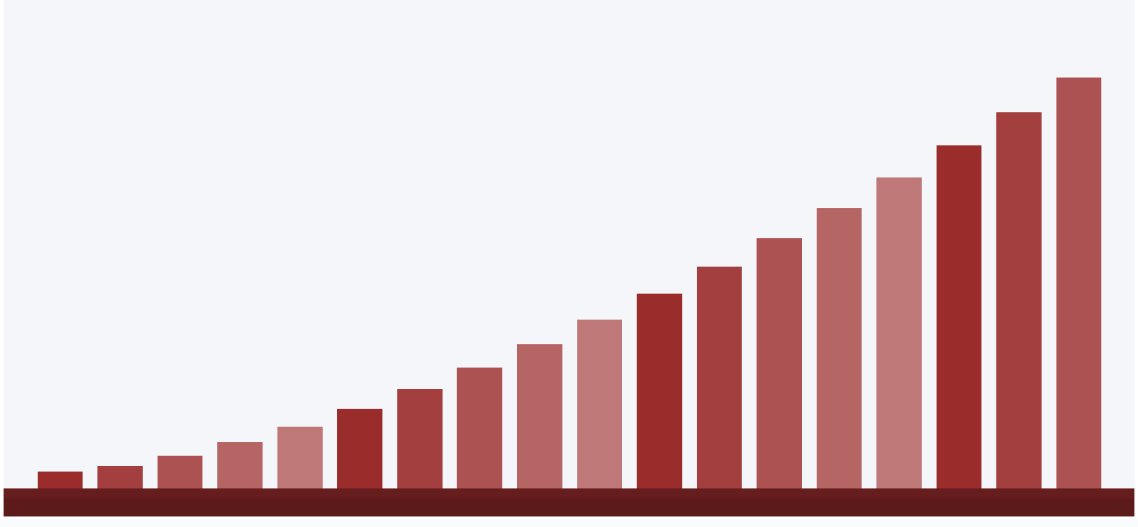
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Behavior Therapy: Principles of Learning**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Behavior Therapy: Principles of Learning to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 24

# Applied Behavior Analysis: Functional Assessment

The ABC model, functional analysis, and the construction of hypotheses about why a behaviour occurs.



BEHAVIORAL THEORIES

## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the origin and the defining commitments of applied behaviour analysis as a discipline and as a practice.
- Distinguish descriptive functional assessment from experimental functional analysis, and state the difference in the strength of the claims each can support.
- Construct an operational definition of a behaviour, and explain why the definition determines the quality of the whole assessment.
- Apply the ABC model to a real behaviour and generate a functional hypothesis from the data.
- Describe the four elements of a functional analysis and the design options for each.
- Explain the distinction between the four functional classes and the three setting, or "mood", demands of behaviour.
- State the difference between descriptive, hypothesis-testing, and experimental functional assessment, and the circumstances in which each is appropriate.
- Plan a functional assessment for a young person, including safety provisions where the behaviour is dangerous.
- Critically evaluate the strengths and limitations of the functional assessment approach, including its ethical dimensions with minors and people who do not use speech.

## Chapter Overview

The clinical question "why does this young person do that?" sounds like an invitation to speculate. In the behavioural tradition it is an invitation to design an experiment.

Applied behaviour analysis — the applied wing of the behavioural framework introduced in Chapter 23 — takes a view of behaviour that is at once narrower and more demanding than any other theory in this book. It is narrower because it confines itself to the behavioural environment of the individual, and it makes no claim about the interior. It is more demanding because it requires the clinician to state, in operational terms, what the behaviour is; to gather data; to generate a specific and testable hypothesis about what the behaviour accomplishes; and then to test that hypothesis against the data. A clinician who cannot state a testable hypothesis has not finished the assessment, and the discipline is unusually clear about this.

The method's power lies in the fact that the hypothesis, once formed, can be checked. The method's liability lies in the fact that a hypothesis that has been checked and found false is more deflating than a guess that was never examined, and the field has a documented history of treating a failed prediction as a challenge to the person rather than as information about the behaviour.

The history of the discipline is inseparable from its history with children. For three decades, the field's flagship intervention was the behavioural treatment of autism, and the controversy that surrounded it — the controversy about aversives, about the training of compliance, about the rights of participants, and about the quality of the research — has shaped every subsequent ethical standard in the discipline and remains the sharpest test of its integrity. A student who reads the applied literature without this history will misread its confidence.

This chapter presents the method in its technical form, because the method is genuinely demanding and it cannot be learned from a description. It then turns to the ethical and cultural questions that the method raises with unusual sharpness, and it concludes with the practice of functional assessment with young people in the settings in which they are actually found.

# 1. Historical Context

## 1.1 The founding of applied behaviour analysis

The discipline named itself in 1968, at a conference held in Washington, D.C., and its founding statement was Donald Baer's paper "Some Dimensions of Applied Behavior Analysis." The definition that was agreed is still the shortest accurate description of the field:

> *Applied behavior analysis is a natural science concerned with the analysis of behavior. It is a basic science with a fundamental characteristic of being a science that is primarily concerned with the analysis of the functional relations between behavior and environment; that is, with the systematic and controlled manipulation of environmental variables and the resulting changes in behavior; and it is a technology for the development of behavior change technologies that are grounded in the science and that are themselves derived from it.*

Three phrases in that definition are worth holding on to, because they mark the field's commitments. "A natural science" claims a place in the scientific family, with the standards of replication, measurement, and falsification that this implies. "Functional relations" commits the field to the analysis of relations rather than to the description of events. And "a technology" commits it to the production of tools that work, with a requirement that the tools be derived from the science rather than assembled from intuition.

The final clause — that the technology must be derived from the science — is the field's most distinctive and most frequently violated standard, and it is the reason the discipline has a well-earned reputation for treating a treatment that has not been validated as an inadequate treatment rather than as an adequate one.

## 1.2 The 1968 criteria for applied research

The same conference produced a set of criteria by which applied research could be assessed, and the list remains one of the most useful instruments in the discipline for evaluating a study — and, by extension, a treatment.

The criteria require that the problem be a behaviour that is important to the individual or to society; that the behaviour have a measurable effect on the environment; that there be a systematic manipulation of some aspect of the environment; that there be a clear relation between the manipulation and the behaviour change; and that the behaviour change be behaviourally defined. The criteria are demanding in a particular way: a study that manipulates nothing is not applied research, and a study that manipulates something and reports a change without establishing the relation is not applied research either.

The 1968 criteria were later revised in 1993 in response to the recognition that the original list was not sensitive to issues of generalisation, of the social validity of the behaviour, and of the participant's quality of life, and the revised criteria add a requirement that the intervention should improve the participant's quality of life, that it should be generalisable to the settings in which the behaviour occurs, and that it should be

acceptable to the participants. The revisions were, in effect, an attempt to answer the ethical criticisms of the preceding two decades.

## 1.3 The Unitarian Position

The 1970s saw a sustained and unusually rigorous theoretical argument within the discipline over the "Unitarian Position" — the claim that a single set of principles about behaviour, expressed at different levels of description, is sufficient to analyse all behaviour, including complex human behaviour.

The position was developed most fully by the "Part I" and "Part II" papers of 1973 and 1974, which argued for a three-level analysis: a level of the functional relations between behaviour and the environment, a level of the relations among the controlling stimuli (the "functional relations" between the environment and the environmental behavior), and a level of the individual's history and private events. The argument was deliberately narrow: it did not claim that behaviour is simple, and it did not claim that complex problems have simple solutions. It claimed that the *analysis* is uniform, and that the apparent differences between the simple and the complex are differences in the complexity of the environmental control, not differences in kind.

The Unitarian Position is significant in this book because it is the most sustained attempt in the discipline to reconcile behaviourism with a richer psychology. It is also the most technical, and a student is not expected to master it. What the student should take from it is the insight that the difference between a simple and a complex case is often a difference in the number and subtlety of the controlling variables, and that the number of variables can be managed if they are specified one at a time.

## 1.4 The controversy with autism

In the late 1960s and early 1970s, a number of applied behaviour analysis programmes delivered what were called behavioural programmes for autistic children, in which a variety of techniques — some of them aversive, some of them involving food or drink as reinforcers, and some of them involving the deprivation of sleep — were used to shape behaviour.

The controversy that followed is the most consequential episode in the history of the discipline, and it is the reason that the field's ethical language is what it is today. Four of the criticisms were substantiated.

**The research was weak.** Studies in this period frequently used designs in which there was no experimental control, in which the outcome was defined in terms that could not be independently verified, and in which the participants were not selected against a control condition. When internal critiques of the early research were published in the 1970s and 1980s, the effect on the field's confidence was severe.

**Some of the techniques were aversive.** The use of contingent aversive stimuli in some programmes was documented, and while most practitioners in the field used the least restrictive procedures available, the incidents were sufficient to shape the field's subsequent ethical standard.

**Informed consent was not obtained in the modern sense.** Parents were asked to consent to treatment, which is not the same as a young person's assent to a specific procedure, and the field later developed distinct and detailed requirements for both.

**The behaviour targeted was sometimes trivial.** Some programmes targeted such behaviours as the lack of a stereotypic movement, on the grounds that any behaviour that interfered with learning should be changed, and the field later became more careful about the difference between a behaviour that is a disorder and a

behaviour that is merely unusual.

The criticisms were not simply an attack from outside the discipline. Many of the sharpest came from behaviour analysts, and the discipline's response was to reform: to require experimental control in applied studies, to develop alternatives to aversive procedures, to formalise assent and withdrawal procedures, to develop the least restrictive procedure standard as a requirement rather than a preference, and eventually to make the participant's quality of life a criterion of success alongside the behaviour change.

The history is not presented here as a matter of settled blame. It is presented because a student who intends to work with young people in behavioural settings will be asked about it, and because the honest answer — that the field made serious errors, that the errors were substantive, and that the discipline's current standards are a direct product of them — is the only answer that is both accurate and professionally usable.

### 1.5 The expansion into other settings

From the late 1970s the discipline expanded rapidly into other applications: classroom behaviour, self-injury, aggression, pica, disruptive behaviour in residential settings, and the teaching of skills to people with intellectual disabilities. The expansion brought the discipline into contact with school systems, hospitals, and courts, and it brought it into contact with the legal and ethical questions that follow from that contact.

The applied behaviour analysis approach to **treatment fidelity** — the systematic measurement of whether the intervention was delivered as intended — is another contribution of the period and is now standard practice in the field, and is a model of professional self-discipline that the wider profession has largely adopted.

## 2. The Method: Four Elements

Every functional assessment in the discipline contains four elements, whatever its setting and whatever its sophistication. A student who cannot name the four has not understood the method.

**The operational definition.** A precise, observable, measurable specification of the behaviour, including the conditions under which it occurs, the conditions under which it does not, and the way it is counted. The definition determines the quality of everything that follows, and no amount of subsequent care compensates for a poor one.

**The observation and recording system.** A structured method of gathering data, with a defined recording unit, a defined recording schedule, and defined rules for recording. The system must be practical enough to be used consistently by the people who will use it, because a measurement system that is not used is not a measurement system.

**The functional hypothesis.** A specific, testable statement about what the behaviour accomplishes, derived from the data and from the interview and observation information, and expressed in a form that can be refuted.

**The test.** The procedure by which the hypothesis is compared with the data, which is either an experimental functional analysis, a hypothesis-testing observational design, or — in the most constrained circumstances — a reasoned judgement that the available information does not permit a test.

### 2.1 Operational definitions

An operational definition specifies the behaviour in terms that any trained observer would agree upon, and it specifies the conditions under which it is counted. Three components are usually required.

**The topography**, or the physical and observable form of the behaviour: what the person does, in terms that can be seen and counted.

**The context**, or the conditions under which the behaviour occurs and is counted: the setting, the time, the activity, and the people present.

**The intensity**, or the measure by which variations in the behaviour are graded, and a stated threshold for what counts.

A worked example clarifies the point. "Daniel is aggressive" is not an operational definition and cannot be used for any purpose. "Daniel hits a peer with an open hand, at a rate of one or more occurrences per five minutes, during unstructured break times in the main corridor, with intensity recorded on a three-point scale from 1 (a single light push) to 3 (a blow causing a peer to fall or requiring adult intervention)" is an operational definition. It can be counted by two observers, it can be compared across weeks, and it can be used to test a hypothesis.

The severity thresholds in the example also matter for a reason that is not merely technical: behaviour that is dangerous requires additional provisions in the plan, and a definition that does not distinguish severity will fail to trigger them.

## 2.2 The ABC model in practice

The Antecedent–Behaviour–Consequence model, introduced in Chapter 23, is the standard observation sheet. Two variants are in common use and they answer different questions.

**The ABC narrative** records the full sequence of events leading up to and following each occurrence of the behaviour, in prose, with the antecedents described at whatever level of detail is available. It is the most informative instrument and the most laborious.

**The ABC tally** records, for each occurrence, the coded antecedent and the coded consequence, in a compact tabular form suitable for a large number of observations. It is less informative per occurrence and much more informative in aggregate.

The components of the antecedent record are typically: the activity in progress, the people present, the location, the time, and any recent change. The components of the consequence record are typically: the adult response, the peer response, the effect on the environment, and any delay before the response.

## 2.3 Generating hypotheses

A **functional hypothesis** is a statement of the form: *This behaviour occurs in order to produce [consequence], and it occurs more often in [setting] and under [antecedent conditions].* Three elements must be present for the hypothesis to be testable: the consequence, the setting, and the antecedent conditions.

The four classic functional classes are the most common hypothesis categories, and they are stated in terms of the consequence produced.

**Attention** — the behaviour is maintained by the delivery of attention, usually social and usually from an adult.

**Escape or avoidance** — the behaviour is maintained by the removal or postponement of a demand or an aversive stimulus.

**Access to tangibles** — the behaviour is maintained by the obtaining of a physical item or activity.

**Automatic reinforcement** — the behaviour produces its own consequence, as in the sensory consequences of repetitive behaviour or of self-injury.

The fifth class, which is often distinguished but is sometimes added to the four, is **social negative reinforcement**, in which the behaviour removes or postpones social interaction, for example ending a conversation.

### 2.4 The three setting demands

Beyond the four functional classes, a second set of hypotheses concerns the *setting demands* that the behaviour accommodates. These are derived from the work of Iwata and colleagues on self-injury and are important because they explain a great deal of behaviour that the four classes do not.

**Automatic** — the behaviour is maintained by its own sensory consequences and is unrelated to the social environment.

**Attention** — the behaviour occurs in the presence of others and is directed at obtaining their attention.

**Escape/avoidance** — the behaviour occurs in response to demands, and terminates or postpones them.

**Attention and escape/avoidance** — the behaviour serves both functions, and it is common in clinical settings where the behaviour is both attention-seeking and a response to a demand.

The value of the setting demands is that they are more directly observable than the functional classes. A practitioner who watches a young person for an hour and can state under what circumstances the behaviour occurs and what happens immediately afterwards has a good foundation for hypothesis testing, even if the interpretation of the underlying function remains tentative.

## 3. Descriptive Functional Assessment

### 3.1 The components

A **descriptive functional assessment** is a structured attempt to identify the antecedents and consequences of a behaviour using indirect and direct observation, interviews, and record review, without manipulating the environment.

Its components are typically:

- **Structured and unstructured interviews** with the young person, the parents, the teachers, and where appropriate the other professionals involved. The interview is not merely a matter of asking what the behaviour is for; in a well-conducted assessment the interviewer is trying to elicit the specific antecedents and consequences, and asking questions of the form "What happens right before?" and "What happens right after?" rather than "Why does he do that?"
- **Direct observation** in the natural setting, using a structured recording system.
- **Record review**, which is frequently the most productive component and is consistently under-used. School behaviour logs, attendance records, exclusion records, incident reports, and nursing notes frequently contain the pattern before anyone has looked for it. The case illustration in Chapter 23, in

which eleven exclusions all fell within a single forty-minute period, is an example of what a record review produces.

- **The ABC analysis**, which organises the observations into the antecedent–behaviour–consequence sequence.

### 3.2 The strengths and the weaknesses

The descriptive assessment is non-invasive, does not require the cooperation of the environment, can be completed quickly, and is suitable for situations in which experimental manipulation would be impractical or unsafe. It is also subject to a severe limitation: **the data are observational, and the observer's presence may change the behaviour**, and more importantly, the descriptive data can support a hypothesis but cannot establish it, because the correlations that the data contain are not themselves the causal relations.

A practitioner who has constructed a plausible hypothesis from descriptive data and then proceeds to design a treatment from that hypothesis without testing it is making a common and consequential error. The hypothesis may be wrong, and if it is wrong, the treatment will be ineffective at best and harmful at worst, because the treatment will be built on contingencies that are not maintaining the behaviour.

The discipline's rule is straightforward: a hypothesis derived from descriptive data is a hypothesis until tested.

### 3.3 The information-gathering interview

The interview deserves separate treatment, because it is where the useful data most often is and where the most frequent errors occur.

The most common error is asking "why" questions. "Why does she hit her teacher?" invites a story, and the story will be a plausible account composed of the interviewee's theories of behaviour and the child's intent, and it will bear little relation to the maintaining contingencies. The productive questions are the concrete ones: what does the child do immediately before? What do the adults do immediately after? How long before an adult arrives? What happens in the ten minutes after? What did the other children do? What was different today?

The second common error is accepting a single informant's account as the account. Parents, teachers, and the young person each observe the behaviour in different settings, with different access, and with different stakes. Disagreement between informants is information: it usually means the behaviour is setting-specific, and the setting-specific pattern is usually where the function is.

The third error is the framing of the question in deficit terms. If the interviewer asks "What is causing this behaviour?", the answer will be a cause. If the interviewer asks "What is happening in the room just before this behaviour happens, and what happens immediately after?", the answer will be data.

## 4. Hypothesis-Testing Observational Designs

Between the descriptive assessment and the full experimental analysis stands a family of designs that test a specific prediction by analysing patterns that already exist in the natural data, without manipulating the environment. They are less rigorous than a functional analysis and considerably more rigorous than a descriptive assessment.

### 4.1 The prediction method

The **prediction method** is the simplest of the designs. The practitioner identifies a specific prediction that follows from the hypothesis — for example, "the behaviour will occur more often in the absence of the teacher than in her presence" — and then examines existing records to test it. The limitation is that the data were not collected for the purpose and may not contain the necessary information.

### 4.2 The ABC correlation method

The **ABC correlation method** analyses the recorded antecedents and consequences to determine whether the behaviour is reliably associated with particular consequences. It is the most useful of the family and the most commonly used. Its logic is direct: if a behaviour is maintained by attention, then it should be followed by attention more often than it is not, and the proportion of occasions followed by attention should exceed the base rate of attention delivery in the setting.

The base-rate comparison is what gives the method its power and is frequently omitted in informal use. A behaviour that is followed by attention on 80 per cent of occasions looks striking, but if the adults in the setting deliver attention to almost every child almost all the time, then 80 per cent is below the base rate and the behaviour may be maintained by something else entirely.

### 4.3 The structured ABC method

The **structured ABC method** goes further by manipulating the recording procedure itself: the recorder is instructed to look for occasions on which the behaviour occurs, and the antecedents and consequences are recorded in a more detailed and more reliable way than the ordinary ABC record permits. It is a substantial improvement on the standard record and it is well suited to the situation in which a hypothesis is already reasonably specific and needs firmer data.

## 5. Experimental Functional Analysis

### 5.1 The logic

A **functional analysis** is a single-case experimental design in which the consequences that follow the behaviour are systematically manipulated across conditions, and the behaviour is measured in each condition. The logic is that if a particular consequence is what maintains the behaviour, then removing that consequence should reduce the behaviour and restoring it should increase the behaviour again.

This is the method that converts a hypothesis into a finding, and it is the method's central contribution. It is also the most demanding, and it requires the full cooperation of the setting.

### 5.2 The standard conditions

A **traditional functional analysis** includes a series of conditions, typically:

- **A play condition**, in which the behaviour should not occur and the environment is arranged to be conducive to it. Behaviour that occurs at high rates in this condition suggests that the behaviour is maintained by its own consequences — that it is *automatically* maintained.
- **An alone condition**, in which the young person is left alone briefly with an observer present. This is a control for the possibility that the behaviour is cued by the presence of others. It is also a condition of

ethical concern, because it involves leaving a young person alone, and it requires the safeguards described in Section 9.

- **A demand condition**, in which demands are made and the opportunity to escape is contingent on the behaviour. Behaviour that increases under demands is consistent with an escape function.
- **A play condition with attention delivered** contingent on the behaviour. Behaviour that increases when attention is delivered is consistent with an attention function.
- **A control condition**, in which the consequence of interest is delivered non-contingently, at random intervals. This is a crucial control, because it distinguishes the *contingent* delivery of a consequence from the mere presence of the consequence, which is what identifies the behaviour as attention-seeking rather than as behaviour that persists in the presence of a stimulus.

The conditions are typically run in an alternating or rotating design, sometimes with a reversal to demonstrate that the effect follows the condition rather than the passage of time.

## 5.3 The practical and ethical difficulties

The traditional functional analysis is often not feasible with a young person, for a set of reasons that are practical rather than theoretical.

**Time.** The design may require multiple sessions across days or weeks, and a young person's pattern of severe behaviour is often unstable over such a period, which compromises the interpretation.

**Safety.** Some behaviours are dangerous, and some of the conditions — particularly the alone condition, and the play-with-attention condition when the behaviour is aggression — carry a risk that requires safeguards.

**Setting cooperation.** The analysis requires that staff, teachers, and parents follow the contingency arrangements precisely, and in a real school or clinic, adherence to a protocol that requires five different staff members to behave in specified ways in specified conditions is a substantial and often unrealistic demand.

**The young person's own behaviour.** A young person who has learned what the conditions mean will behave differently across them, and the very behaviours that are most in need of assessment are the ones most likely to be affected by the assessment.

**The absence of a natural setting.** Some of the conditions cannot be arranged in the settings in which the behaviour actually occurs.

## 5.4 Brief and practical functional analyses

The literature contains a series of modified designs developed in response to these difficulties, and a student should know them because they are what is actually used.

The **brief functional analysis** reduces each condition to a short block, typically five minutes, and reduces the number of conditions, often to three or four. It is the workhorse of contemporary clinical practice and it can be completed in a single session in most cases.

The **practical functional analysis** is a specific and widely used protocol, developed for use in naturalistic settings with typically fewer and briefer conditions, and designed so that the analysis can be conducted by caregivers or by school staff rather than only by a specialist.

The **synthesis of multiple indirect and direct evidence** approach, sometimes called the **indirect plus direct** method, is a common strategy in which a hypothesis is first narrowed using interviews and record review, and then tested with a brief direct analysis. The indirect evidence is used to select and design the direct conditions, which is a legitimate use of indirect data — provided the test is still experimental.

### 5.5 What the functional analysis does not tell you

A functional analysis establishes which consequence maintains a behaviour in a particular setting and under particular conditions. It does not establish why the behaviour developed, what the young person means by it, or what should be done about it. A functional analysis is an answer to a narrow question — *what does this behaviour do?* — and a treatment plan that extends from it is making a step that requires a judgement of a different kind.

This is a point on which the behavioural and the humanistic traditions can be usefully joined, and it is worth putting to students directly: the functional analysis is a genuinely valuable and rigorous tool, and it is capable of establishing things that no other method in this book can establish, and it says nothing whatever about the meaning the behaviour serves, and both of these are simply true.

## 6. Functional Assessment with Children and Adolescents

### 6.1 Setting-specificity

The most important developmental observation is that the function of a behaviour in a young person is frequently **setting-specific**. A young person may be calm in the counselling room and dysregulated in the classroom; cooperative with one teacher and oppositional with another. A behaviour that is maintained by escape in one setting and by attention in another cannot be addressed by a single plan, and a practitioner who assesses only one setting will produce a plan that works in the assessed setting and not in the others.

The practical implication is that the assessment should be conducted in the setting where the behaviour occurs, by someone with access to it, and that the referral question should be reformulated accordingly. A referral that says "he is aggressive" should be replaced by "he is aggressive in the corridor at break, and here is the log showing when."

### 6.2 The developmental context of the consequences

The consequences available to a young person differ in important ways from those available to an adult, and the assessment must take account of them.

**The contingency density is lower.** Young people's environments are more structured, with more adult-determined consequences, and often fewer opportunities for the behaviour to produce an effect. A behaviour that would be reinforced by a glance in an adult setting may be entirely unreinforced in a classroom with six other pupils and a teacher who is trying to teach.

**The escape contingencies are typically more powerful.** School demands are aversive for many young people, and the escape behaviour is often strongly reinforced because the relief of the demand is immediate and large.

**The social consequences are peer-dominated.** For most adolescents, the social consequences of a behaviour outweigh the adult ones by a considerable margin, and a functional assessment that examines

only adult responses will miss the functional relevance of peer approval entirely.

**The developmental appropriateness of the consequence changes the analysis.** A consequence that would be strongly reinforcing at fifteen is nearly irrelevant at six, and the value ordering that the young person reports should be treated as a clinical datum rather than as a set of preferences to be indulged.

### 6.3 The involvement of the family and the school

Applied behaviour analysis in a school or clinic is never a solo activity. The assessment happens in a system, the data are gathered by people in that system, and the plan requires their continued adherence. Two practical consequences follow.

First, the assessment should be designed so that the people who will implement the plan are involved in designing it. A plan designed in an office and delivered to a classroom is a plan with a low probability of being implemented as written, and the low fidelity will be misread as the failure of the intervention.

Second, the data should be collected by the people who will implement the plan, or at least by a person who works alongside them, because the fidelity of the data collection determines the validity of the whole process.

### 6.4 Provisional plans and the treatment integrity question

Where a full functional analysis is not feasible, the discipline permits a **provisional treatment** based on a descriptive assessment, on the explicit condition that the hypothesis is treated as provisional and is revisited. The practice of provisionally implementing the most likely hypothesis while continuing to collect data is a reasonable clinical compromise, and it is only reasonable when the data collection continues.

The risk that must be named is the one that runs through the whole discipline: a provisional hypothesis that is never tested becomes a settled assumption, and the behaviour that does not respond is attributed to the young person's characteristics rather than to the hypothesis. The clinical safeguard is a pre-agreed review point at which the question "has the hypothesis been tested?" is asked explicitly.

## 7. Evidence and Research Support

### 7.1 The status of the evidence for the method

The evidence for applied behaviour analysis is unusual in structure. The method itself — the logic of the functional analysis — is not an intervention and is not directly subject to randomised evaluation. What has been evaluated, extensively, are the interventions that the method produces, and the overall conclusion of the systematic reviews is that behavioural interventions based on a functional assessment are effective across a wide range of behaviours and settings, with effect sizes that are among the largest reported in the psychological literature.

The more useful evaluation for a student concerns the components. Three findings recur.

**Function-based interventions outperform non-function-based interventions.** The reviews consistently find that interventions built on a functional analysis produce larger and more durable effects than interventions built on a behaviour-reduction logic. This is the single most important empirical finding in the applied literature and it is exactly the finding the theory predicts.

**The functional analysis itself is not always necessary.** For many common problems, a well-conducted descriptive assessment combined with a clear rationale produces results comparable to those from a full experimental analysis, and the reviews find no reliable superiority of the experimental design for all problems. This is an important and humbling finding, and it should temper the discipline's enthusiasm for its own most elaborate technique.

**The quality of the implementation determines the effect.** Studies that measure treatment fidelity and find low implementation fidelity report null results that reflect the implementation rather than the intervention. The field's insistence on fidelity measurement is, in effect, a refusal to draw conclusions from data it knows to be contaminated.

### 7.2 The methodological contribution

The methodological contribution of applied behaviour analysis to the wider profession is considerable, and three items deserve naming.

**Single-case experimental designs.** The logic of demonstrating a functional relation in an individual case, using repeated measurement across conditions, is a genuine and rigorous methodology, and its influence extends far beyond the discipline.

**Fidelity measurement.** The systematic measurement of whether an intervention was delivered as intended, and the reporting of that measurement alongside the outcome, is a standard that the whole profession has adopted.

**Operational definitions as a professional discipline.** The insistence on defining behaviour in terms that another person could count, and the practice of recording those definitions, has made it possible for professionals in different settings to talk about the same behaviour. The language of applied behaviour analysis is now the shared language of much of clinical practice, and it is a more precise language than the one that preceded it.

### 7.3 Limitations of the evidence

Three limitations should be stated.

**The literature is dominated by specific target behaviours**, many of them in institutional settings, and the generalisation to the kinds of problems that dominate counselling practice in the community is not straightforward.

**Many studies lack independent replication**, and the fields in which the discipline has been most active have attracted more scrutiny than replication.

**The outcome measures are almost always behavioural**, and the measures of the young person's quality of life, of the family's experience, and of the durability of the change in naturalistic settings are less systematically reported, and the 1993 revisions of the applied research criteria were directed at precisely this gap.

## 8. Strengths and Limitations of the Functional Assessment Approach

### 8.1 Strengths

**It is testable.** The distinguishing feature. Most of the profession's diagnostic formulations are not capable of being refuted by data, and a formulation that cannot be refuted cannot be evaluated.

**It is practical.** The method produces a specific, implementable plan, and it produces the data by which the plan can be monitored.

**It is transparent.** The plan can be written down, explained to the young person and the family, and handed to a teacher.

**It treats the young person as an individual.** A functional analysis produces an explanation that is specific to one person in one context, which is what a young person is entitled to.

**It generates the alternative behaviour from the analysis.** The most useful feature of a function-based approach is that the replacement behaviour is derived from the function, and is therefore likely to be effective and is likely to be taught in a way the young person can use.

### 8.2 Limitations

**It is only as good as the data.** The method's precision is a promise it cannot always keep. Poor operational definitions, low-fidelity recording, and observers who are not trained produce confident conclusions from unreliable data.

**The hypotheses can be narrow.** The four functional classes are a convenient taxonomy, and the pressure to fit a case into one of them is real. Behaviour that serves multiple functions, or a function that is not in the taxonomy, is a persistent source of error.

**It can be used to justify control.** This is the serious criticism, and it has been made from inside the discipline as well as outside it. A functional analysis of a young person's protest behaviour can produce a technically correct plan for extinguishing the protest, and the technical correctness of the plan says nothing about the justice of it. The discipline's answer is that the ethical criteria govern the choice of targets, not merely the choice of technique, and that the young person's assent and quality of life are criteria of success. That answer is right in principle and requires constant attention in practice.

**It under-accounts for context.** A functional analysis conducted in a contrived setting may not generalise, and the setting demands of the analysis are themselves a simplification of a complex situation.

**It requires resources.** The method asks for trained observers, structured recording, stable environments, and the cooperation of several people. In a service with high caseloads and low continuity, it is expensive, and the honest practitioner must sometimes say so.

## 9. Cultural and Ethical Considerations

### 9.1 The young person's own account

A young person who is being assessed is usually the last to be consulted and the first to be surprised. The practice of functional assessment with minors has been the subject of sustained criticism, and the criticisms are worth taking seriously.

**The asymmetry of power.** The assessment is conducted by an adult with authority over the young person, and it is directed at changing the young person's behaviour for the benefit, as often as not, of adults and institutions. The young person may have no understanding of why the assessment is happening, and may not

be in a position to decline it.

**The young person is not a passive object of analysis.** A young person who is being observed in a corridor knows they are being observed, and will behave accordingly, and the behaviour the observer records will be shaped by the observation. A young person who knows that the analysis is looking for the function of a behaviour can be, in the most benign sense, strategic about it.

**Assessment should include the young person's own account.** Where the behaviour has a communicative function, the young person's own description of it is the most direct evidence of the function available, and it should be sought as early as possible, in a form that does not require the young person to explain themselves fluently. A young person who says "I do it so they send me home" has told you the function, and no amount of ABC recording will improve on it.

## 9.2 Assent, withdrawal, and the limits of the method

The discipline's ethical requirements, which are more developed than those of most of the profession, are worth stating in full because they are a model of how such requirements should work.

**Informed consent and assent.** The parent or guardian consents to the assessment; the young person assents, and assent requires the capacity to decline. For a young person who cannot understand the procedure, the assessment should not proceed in its full form.

**The right to withdraw.** The young person may withdraw at any point, and the consequences of withdrawal are stated in advance, so that withdrawal is genuinely available and not punished.

**The least restrictive effective procedure.** The least intrusive method that can produce the required result is used, and the consideration of alternatives is documented.

**Aversive procedures are not used.** The contemporary discipline does not use contingent aversive stimuli as a planned component of treatment.

**Protection from harm.** Procedures are designed so that they do not produce harm, and the conditions involving deprivation or isolation are used only under specified safeguards and with a full risk assessment.

**The participant's quality of life as an outcome.** The 1993 criteria require that the intervention improve the participant's quality of life, and this is the most novel and most important of the requirements, because it makes the young person's own experience of the treatment a criterion of its success rather than an afterthought.

## 9.3 The conditions that require particular care

Two conditions in the traditional functional analysis require specific safeguards and should never be run without them.

**The alone condition** involves leaving a young person alone, which for a young person with a history of abuse, with separation anxiety, or with self-injury is a substantial risk. The condition should be brief, should be conducted with a clear observation protocol and an immediate response plan, and in many cases should be replaced by a condition that is observationally similar without requiring separation.

**The demand condition** requires the deliberate presentation of demands to a young person, and where the young person has trauma-related responses, presenting demands in an unrepresentative way can produce a re-experiencing. The demands should be ordinary, should be brief, and the young person should be able to end the condition.

**The attention condition** requires a planned interaction in which a young person's behaviour is met with attention. Where the behaviour is aggression, this condition requires a specific safety protocol, and where the young person has been harmed by adults who were supposed to protect them, the condition can itself be a trigger. This is one of the points at which the applied literature and the trauma literature meet, and the trauma-informed position is that the attention condition with a survivor of adult maltreatment should be modified or replaced.

### 9.4 Cultural considerations

**The behaviours that get referred.** The referral pattern is itself culturally patterned, and a behavioural analysis conducted in isolation from the referral pattern can miss the context. A behaviour referred because it is disruptive in a classroom may be a behaviour that is a form of communication about something the family and the community have already identified as a problem and that the school has not.

**The function is a cultural attribution.** The same behaviour has different functions in different settings, and the practitioner who transfers a hypothesis from one setting to another without re-testing it is making a cultural error as well as a technical one.

**The contingencies of the wider community.** The reinforcement structure that the family operates within — the ways in which respect, knowledge, and belonging are distributed — will determine whether a clinic-based plan survives its contact with home. A plan that competes with the family's own logic of reinforcement will be defeated by it, and the defeat will be blamed on the family.

**The professional authority of the assessor.** In many settings, the assumption that an outside professional has the standing to set a young person's contingencies is itself a cultural assumption. A practitioner who arrives with a plan that has been designed without any consultation of the family is likely to encounter resistance that is rational and that should be understood on its own terms.

## 10. Case Illustration: The Young Person Who Became Quiet

**Presentation.** Priya is fifteen and attends a school that has asked for a behavioural assessment because, in the words of the referral, "she has become withdrawn and is not engaging." The school's concern is framed as a management problem: Priya is now spending her breaks in the library rather than in the corridor, she is declining to join in a weekly group session that her form tutor has organised, and she has stopped attending the school football team, which she had played in the previous year.

Her mother reports a different history. She says that Priya became "a different person" after a school trip in the previous March, that she now will not sleep in her own room, that she has started spending her nights in her mother's bedroom, and that she has twice been found in the bathroom at three in the morning.

**The first hypothesis, and why it was insufficient.** The first functional hypothesis, generated from the school's information, is that Priya's behaviour is escape-maintained: she avoids the corridor because of a peer, avoids the group session because of social anxiety, and avoids football because of the heat. The hypothesis is not unreasonable, and the evidence for it is thin. The problem is a methodological one and it is instructive: the hypothesis has been generated from the referral, and the referral is a description of a change in behaviour, not a record of the circumstances in which the behaviour occurs.

**The descriptive assessment.** The assessment proceeds in four stages.

*Record review.* The school incident log, the attendance register, the behaviour log, and the pastoral notes are gathered. The pattern that emerges is that Priya's attendance rose slightly over the year while her participation in anything requiring her to be approached by a group of peers fell sharply, and that the specific date of the change in the pastoral notes is the date of the trip. The attendance data are important because they are the kind of data that would have been overlooked: Priya is not absent. She is present and she is participating in nothing.

*Interviews.* The mother is interviewed with the specific questions — what happens right before Priya comes out of the bathroom at three in the morning, what happened on the trip. The form tutor is interviewed about the corridor at break and the group session. Priya is interviewed, and this is where the assessment is most easily and most consequentially mishandled, because the obvious move is to ask her why she has stopped playing football, and the answer she gives is a non-answer that will be read as evasion.

*The young person's own account.* Priya is asked what has changed, and she says, "Everyone was so happy about it that I couldn't say anything." Asked what she means, she says that there were photos, and that someone said they would send the photos around, and that "now people look at me different." She is not willing to say more. This is the single most informative datum in the entire assessment and it is the datum that a purely behavioural account of a sixteen-year-old's withdrawal would never have obtained.

*Direct observation.* The counsellor arranges to be present in the corridor, the library, and the changing room at break over several days, recording the antecedents and the consequences of Priya's behaviour in the ABC format, and observing who approaches her and what follows her refusals.

**The ABC data.** The record is consistent. Priya's behaviour — spending her break in the library, declining the group session, declining football — is preceded by a consistent antecedent: an approach by one or more peers, sometimes with a question, sometimes with a comment. It is followed by a consistent consequence: the peers leave. In the case of the group session, she is called out of it; the coach reports that he is "used to it now."

**The revised hypothesis.** The behaviour is maintained by **escape from peer attention and from situations in which attention might be drawn to her**, and the escape is reliably produced because the peers have learned that insisting has no effect. The school has inadvertently reinforced the withdrawal by accepting it. The library has become a place where nothing is asked of her.

The hypothesis is *not* that Priya is depressed, and it is *not* that she is anxious in the general sense, and it is *not* that she needs a group session. The hypothesis is that she has learned that withdrawing is effective, and that the learning has been reinforced by a well-intentioned school, and that the reinforcement is now sustaining a pattern that is making her worse.

**The intervention.** Four elements.

*The data collection stops.* This is deliberate and it is the element that is hardest to sell, because it looks like doing nothing. The ABC recording was in the corridor and the library; it stops, because the young person has a right not to be observed for the purpose of being changed.

*The contingency is restructured.* The form tutor and the pastoral team agree a protocol: the group session becomes optional, with a stated alternative activity of equal social value; the football coach's invitation changes from a direct request, which invites a visible refusal, to an open, drop-in arrangement, so that attending is no longer an event that can be declined; and the library is staffed by a person rather than being unsupervised, so that the peers' attention can be redirected without a scene.

*No one asks her to explain.* The counsellor does not ask Priya to talk about the trip, does not ask to see the photographs, and does not ask her to confront anyone. The assessment produced a clear account of the contingencies; the contingencies are what is being changed, and the young person is not required to produce a narrative to justify the change.

*Safety and referral.* The night-time waking, the sleeping in the mother's bed, and the bathroom episodes are assessed as a separate matter, and the counsellor speaks to the mother about what the young person may be experiencing and what the mother can do, and the mother is supported in arranging a clinical referral for the trauma-specific assessment described in Chapter 38. The school counsellor and the trauma clinician coordinate, with the mother's and Priya's knowledge.

**The outcome.** Over the following two months, Priya's participation is measured with the school's ordinary registers, which is itself part of the plan, since a plan that generates a special measurement that stops when the plan ends is a plan that cannot be evaluated. Participation in the drop-in football increases. The group session remains declined, and this is accepted, because the hypothesis was about the contingencies and not about the specific activity. The night-time waking continues, and it is not a school matter.

**The interpretation.** This case demonstrates the central methodological point of the chapter: a functional analysis of Priya's behaviour would have produced a *correct* analysis of her school withdrawal and a plan for managing it, and the plan would have been appropriate, and it would have been insufficient. The behaviour at school was a coping response to something that happened elsewhere. The behavioural method was the right tool for the school question, and the school question was not the whole question, and a practitioner who had run the analysis and delivered the plan and stopped would have treated the part of the problem that was visible as though it were the problem.

The case also illustrates the point about the young person's own account. Priya's single sentence about the photographs contained the functional information that the ABC data would otherwise have taken six weeks to approach, and it contained the referral information that no behavioural analysis would have produced. The assessment that included it was better than the assessment that did not, and the reason is not that behavioural data are inferior; it is that behavioural data are data about behaviour, and the meaning of the behaviour was located elsewhere, in a photograph, on a trip, in a group of peers.

## 11. Key Terms

### Key Terms at a Glance

- **Applied behaviour analysis (ABA)** — The discipline concerned with the analysis of functional relations between behaviour and the environment and with the development, from that analysis, of technologies for changing behaviour.
- **The 1968 criteria** — The five conditions that applied research must satisfy: an important problem, a measurable behaviour, a systematic environmental manipulation, a clear relation between manipulation and behaviour change, and a behaviourally defined outcome.
- **The 1993 revisions** — The additions of quality-of-life, generalisability, and social-validity requirements to the applied research criteria.
- **Unitarian Position** — The claim that a single set of principles, expressed at different levels of description, is sufficient to analyse all behaviour including complex human behaviour.
- **Functional relation** — A statement of covariation between behaviour and specified environmental conditions, established under experimental control.
- **Operational definition** — A precise, observable, measurable specification of a behaviour, including its context, its recording unit, and its intensity threshold.
- **Topography** — The physical and observable form of a behaviour, as distinct from its function.
- **The ABC model** — The recording framework for the Antecedent, the Behaviour, and the Consequence of a behaviour.
- **ABC narrative** — A prose record of the full sequence of events surrounding each occurrence of a behaviour.
- **ABC tally** — A compact tabular record of coded antecedents and consequences, used for aggregate analysis.
- **Functional hypothesis** — A specific and testable statement of what a behaviour accomplishes, formulated with the consequence, the setting, and the antecedent conditions.
- **Descriptive functional assessment** — A structured assessment using interviews, observation, and record review, without manipulating the environment; it can generate a hypothesis but cannot confirm one.
- **Hypothesis-testing observational design** — A design that tests a specific prediction using patterns in naturally occurring data, without experimental manipulation.
- **Prediction method** — The simplest hypothesis-testing design, in which a prediction derived from the hypothesis is checked against existing records.
- **ABC correlation method** — A design that examines whether the behaviour is reliably associated with particular consequences, compared against the base rate of those consequences in the setting.

## Key Terms at a Glance (continued)

- **Structured ABC method** — A design in which the recording procedure is deliberately modified to increase the reliability and detail of the antecedent and consequence data.
- **Base rate** — The frequency with which a consequence occurs in the setting independently of the behaviour; the comparison against which any contingency effect must be assessed.
- **Experimental functional analysis** — A single-case design in which the consequences following the behaviour are manipulated across conditions in order to test the functional hypothesis.
- **Traditional functional analysis** — The full experimental design with play, alone, demand, attention, and control conditions.
- **Brief functional analysis** — A shortened version of the experimental design, typically five-minute conditions, that can be completed in one or two sessions.
- **Practical functional analysis** — A modified experimental design developed for naturalistic settings and for use by caregivers and school staff.
- **Control condition** — A condition in which the consequence of interest is delivered non-contingently, to distinguish contingent delivery from mere presence.
- **Attention function** — The hypothesis that a behaviour is maintained by the delivery of social attention from others.
- **Escape or avoidance function** — The hypothesis that a behaviour is maintained by the removal or postponement of a demand or an aversive stimulus.
- **Access-to-tangibles function** — The hypothesis that a behaviour is maintained by the obtaining of a physical item or activity.
- **Automatic reinforcement** — Maintenance of a behaviour by its own consequences, without reference to the social environment.
- **Setting demands** — The automatic, attention, escape/avoidance, and combined demands that describe the circumstances under which a behaviour occurs, as distinct from the functional classes.
- **Play condition** — The condition in which the environment is arranged for the behaviour to occur freely, used to identify automatically maintained behaviour.
- **Alone condition** — The condition in which the young person is briefly left alone with an observer present; it requires explicit safeguards.
- **Demand condition** — The condition in which demands are made and escape is contingent on the behaviour, used to identify escape-maintained behaviour.
- **Provisional treatment** — A treatment implemented on the basis of a descriptive assessment, with the hypothesis explicitly treated as provisional and subject to continued testing.

**Key Terms at a Glance (continued)**

- **Treatment fidelity** — The systematic measurement of whether an intervention was delivered as intended, reported alongside the outcome.
- **Single-case experimental design** — A design establishing a causal relation in an individual case through repeated measurement across conditions, with designs denoted ABAB, A-B-A-B, and BCAB.
- **Interobserver agreement** — The percentage of occasions on which two independent observers code the same behaviour identically; the basic reliability check of behavioural measurement.
- **Least restrictive effective procedure** — The requirement that the least intrusive method capable of producing the required result be used, with the consideration of alternatives documented.
- **Assent** — The young person's own agreement to participate, which requires the capacity to decline and which is distinct from the parent's consent.
- **Session-based data** — Data recorded in discrete, defined observation periods, as distinct from continuous recording.
- **The three-term contingency** — The behavioural relation in which a discriminative stimulus, a behaviour, and a consequence are specified together.
- **The behaviour analyst's ethical obligation to the participant** — The requirement that the participant's assent, welfare, and quality of life are criteria of the success of the intervention.
- **Outcome measurement** — The specification in advance of what will be measured, how, and over what period, in a form that can be compared across participants and across time.

## 12. Chapter Summary

Applied behaviour analysis is a discipline and a practice, and the practice is the disciplined application of the analysis of functional relations to the behaviour of a specific individual in a specific setting. The founding definition of 1968 committed the field to being a natural science, to the analysis of functional relations, and to deriving its technologies from the science rather than from intuition, and the 1968 and 1993 sets of applied research criteria remain the sharpest instruments available for evaluating behavioural treatment research.

The method has four elements, and the name of each one is worth remembering: the operational definition, which fixes precisely what is being counted and how; the observation and recording system, which gathers the data; the functional hypothesis, which states what the behaviour accomplishes in a testable form; and the test, which checks the hypothesis against the data. A clinician who can produce a hypothesis but not a test has not completed an assessment, and the discipline is unusually clear about this.

The hierarchy of methods runs from the descriptive assessment, which gathers information and generates hypotheses but cannot confirm them; through the hypothesis-testing observational designs, which examine predictions in naturally occurring data; to the experimental functional analysis, which manipulates the consequences across conditions and can establish a functional relation. The chapter described the standard conditions, the practical and ethical difficulties of each, and the brief and practical modifications that do

most of the clinical work in contemporary settings.

The four functional classes — attention, escape or avoidance, access to tangibles, and automatic reinforcement — and the three setting demands describe the circumstances under which a behaviour occurs and what it produces. The chapter emphasised that a functional analysis establishes what a behaviour *does* and says nothing whatever about what it *means*, and that both of these statements are simply true, and that the behavioural and humanistic traditions are joined at this point rather than divided by it.

With young people, the assessment must take account of the setting-specificity of function, the density and type of the contingencies available to them, the dominance of peer consequences, and the involvement of the adults who will carry the plan. Where a full analysis is not feasible, a provisional treatment is legitimate provided the hypothesis is treated as provisional and the review point is agreed in advance.

The evidence supports the interventions the method produces, and the most important finding in the reviews is that function-based interventions outperform non-function-based ones — which is exactly what the theory predicts. The experimental design is not always necessary, and the discipline's own reviews are candid about this. The methodological contributions — single-case designs, fidelity measurement, and the discipline of operational definition — have influenced the whole profession.

The ethical analysis is the sharpest in the book, because the field's history with children is genuinely troubled. The honest account is that serious errors were made, that they were substantive, and that the discipline's current standards are a direct product of them. The requirements that follow — assent and the right to withdraw, the least restrictive effective procedure, the protection of the young person during the alone and demand conditions, the treatment of the participant's quality of life as an outcome, and the inclusion of the young person's own account of the behaviour — are the field's own answer, and they are a model for the rest of the profession.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. State the founding definition of applied behaviour analysis, and explain the significance of each of its three commitments.
2. State the five 1968 criteria for applied research, and the three requirements added in 1993.
3. Explain the Unitarian Position and state what a student is expected to take from it.
4. Summarise the four criticisms of the early applied behaviour analysis research with autistic children, and state how the discipline responded.
5. Name the four elements of a functional assessment and explain the function of each.
6. Construct an operational definition for a behaviour of your choosing, and explain the purpose of each of its three components.
7. Distinguish the ABC narrative from the ABC tally and state when each is preferable.
8. State the four functional classes and the three setting demands, and give an example of a behaviour for which the two schemes would suggest different interventions.
9. Explain why a descriptive assessment cannot confirm a functional hypothesis.
10. Describe the three question types that produce usable information in an interview, and the three errors that are most commonly made.
11. Describe the prediction method, the ABC correlation method, and the structured ABC method, and state the role of the base rate in the second of them.
12. Describe the standard conditions of a traditional functional analysis and state what each is designed to detect.
13. Explain the role of the control condition in a functional analysis.
14. State the four practical and ethical difficulties of the traditional functional analysis.
15. Distinguish a brief functional analysis from a practical functional analysis.
16. Explain what a functional analysis does not establish.
17. Explain the significance of setting-specificity for the assessment of a young person's behaviour.
18. Describe the four ways in which the developmental context changes the contingencies available to a young person.
19. Explain the requirement to involve the adults who will implement the plan in designing the assessment.
20. State the circumstances in which a provisional treatment is appropriate, and the safeguard that must accompany it.
21. Summarise the three recurrent findings in the reviews of the applied evidence.
22. State the three methodological contributions of the discipline to the wider profession.

**Review and Discussion Questions (continued)**

23. State the seven ethical requirements of the discipline's practice with minors, and explain the significance of the quality-of-life requirement.
24. Explain the three safety considerations that apply to the alone, demand, and attention conditions.
25. State three ways in which a functional assessment can be culturally biased, and describe the safeguard for each.

## 14. Practice Activities

**Practice and Application**

### Activity 24.1 — Operationalising a Behaviour (60 minutes, individual)

Take a behaviour you have observed in placement and write three versions of its operational definition: the first at a level you would consider obviously too vague, the second at a level you think is defensible, and the third at a level of precision that would be impractical to record. Then write the recording unit for each and say which is the most likely to produce reliable data and why. The activity teaches the central lesson of the method, which is that the definition is not a preliminary to the work but a large part of the work itself.

### Activity 24.2 — The Record Review (90 minutes, individual or pair)

Obtain a school or service record for a period of at least a term — exclusion logs, incident reports, attendance records, nursing notes, or counselling session records — and analyse it for pattern rather than for totals. Plot the occurrences against time of day, day of week, and location, and look for the anomalies. Most records contain a pattern that nobody had noticed, and the exercise is the cheapest and most under-used component of a functional assessment. Report the pattern in three sentences and state what you would observe next.

### Activity 24.3 — The ABC Hypothesis Table (75 minutes, small group)

In groups of four, take a behaviour from a placement and complete the ABC analysis for a single observed occurrence. Then, working as a group, write the functional hypothesis in the required form — consequence, setting, and antecedent conditions — and then write the specific prediction that the hypothesis makes about the data. Finally, write the single observation that would refute the hypothesis. The final step is the one that separates a hypothesis from a guess, and groups consistently find it harder than the earlier steps.

### Activity 24.4 — A Provisional Plan with a Review Point (60 minutes, individual)

Design a provisional intervention for a behaviour for which a full analysis is not feasible. Write the plan, the data collection procedure, the fidelity check, and — crucially — the pre-agreed review point, with the date and the specific question to be asked at it: "has the hypothesis been tested?" Then write what you will do if the answer at the review point is no. The last part is the part that distinguishes a working professional from a person who has found a plausible explanation and stopped looking.

## 15. Annotated Reading Pathways

**For the discipline.** Read Baer's founding paper and the 1973 and 1974 Unitarian Position papers in excerpt, and read a current introductory text on applied behaviour analysis. Then read the 1993 revision of the applied research criteria, which is short and which reframes the whole field.

**For the method.** Read the functional analysis literature from the 1980s, in which the standard design was established, and then read the brief and practical analysis literature, which is what you will actually use. A student who reads only the original literature will have the impression that functional analysis is a laboratory procedure.

**For the ethics.** Read the discipline's ethical guidelines in full, and read the critical literature on the history of the field's work with children. The pairing is the preparation for the consent and assent work in Part VIII and for the forms in Appendix A.

**For the evidence.** Read a systematic review of function-based interventions and a Cochrane review, and read a treatment fidelity study alongside them. The difference in the framing of the two is instructive.

**For the practice.** Read a single-case design handbook, and design a study of a real behaviour in a placement using a reversal or withdrawal design. Designing one is the fastest way to understand what the discipline's discipline actually consists of.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Applied Behavior Analysis: Functional Assessment** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the ABC model, functional analysis, and the construction of hypotheses about why a behaviour occurs. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Kofi is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Kofi's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Kofi explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated

questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Kofi chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Kofi has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Kofi can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Applied Behavior Analysis: Functional Assessment

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Applied Behavior Analysis: Functional Assessment** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Applied Behavior Analysis: Functional Assessment** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

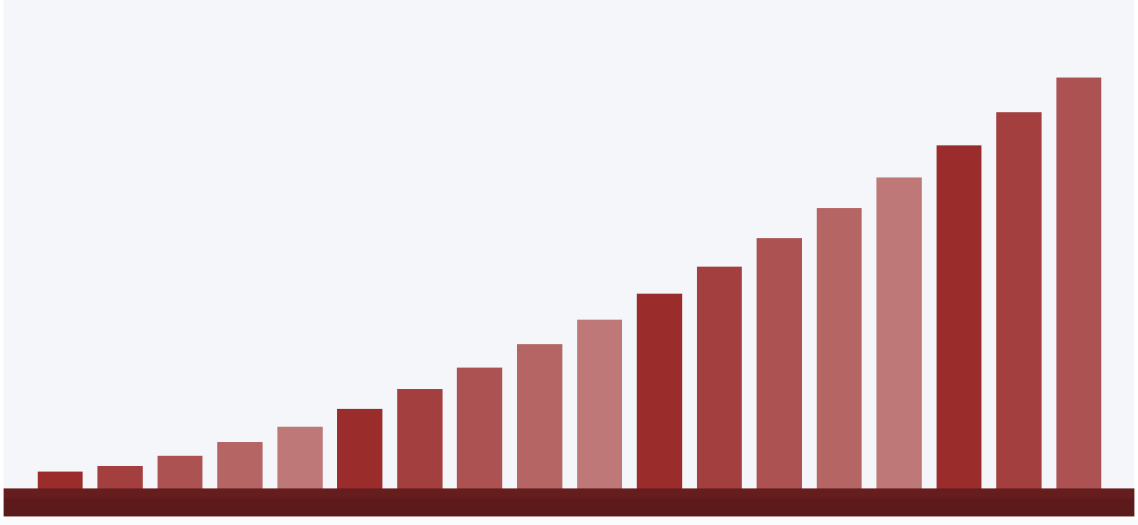
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Applied Behavior Analysis: Functional Assessment**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Applied Behavior Analysis: Functional Assessment to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 25

# Exposure, Modeling, and Behavioral Skills Training

The treatment of avoidance, fear, and deficit through systematic desensitisation, observation, and rehearsal.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the origin of the exposure therapies in the learning theory of fear, and describe the evidence that overturned the psychoanalytic account of phobia.
- Distinguish the exposure-based procedures from the behavioural therapies that preceded them, and state the single principle on which all of them rest.
- Describe the common elements of contemporary exposure protocols, including the identification of the feared stimulus, the construction of a hierarchy, and the planning of the first session.
- Explain the varieties of exposure — in vivo, imaginal, interoceptive, and virtual — and state the indications and limitations of each.
- Describe the rules of exposure practice and explain the rationale for each.
- Explain the "first day" design and justify its use with young people.
- Describe modelling, the conditions under which it works, and the ways in which it can be adapted for young people.
- Describe behavioural skills training, its four components, and its relation to the cognitive and behavioural traditions.
- Apply exposure and skills training to a case involving a young person, and evaluate the ethical constraints on the work.

## Chapter Overview

The exposure therapies are the most successful behavioural interventions in the history of the profession, and the most successful single psychological treatment for anxiety disorders. They emerged from a specific theoretical dispute, and understanding the dispute is the key to understanding the treatment.

The psychoanalytic account of phobia held that the phobia was a symbol — an intrapsychic conflict displaced onto a stimulus that happened to be present. The learning account held that the phobia was an *acquired* fear, produced by the pairing of the feared stimulus with aversive events, and that the fear was maintained because the person avoided the stimulus and thus never learned that it was safe. The treatment followed directly from the account: the fear would be unlearned by the systematic re-presentation of the feared stimulus without the aversive event, in conditions in which avoidance was prevented.

For fifty years, the two accounts produced different treatments and different predictions. The decisive evidence came from the experimental comparison of exposure and psychoanalytic interpretation in the treatment of phobias, and from the observation that a behavioural treatment producing no insight whatever could abolish a phobia in a small number of sessions. The insight that the field took from this was methodological rather than theoretical: a treatment that works changes the standing of the theory that explains it, and the theory of phobia had been wrong.

The principles are simple. The feared stimulus must be presented. The contingency between the presentation and the aversive event must be broken, so that the person learns that the feared outcome does not occur. The avoidance must be prevented, because avoidance is what maintains the fear. The presentation must be repeated, because one success is not learning. And the work must be done by the person themselves,

because a fear that is endured in the presence of a therapist is not the same fear as one endured alone.

This chapter presents the exposure therapies, the modelling and skills-training methods, and the adaptations that young people require, together with the ethical constraints that the work imposes. It is a chapter in which the technical detail matters more than usual, because the details are what make the treatment safe.

# 1. Historical Context

## 1.1 Desensitisation and the imagery dispute

Joseph Wolpe's *Psychotherapy Reciprocity* appeared in 1958, and the technique it described — systematic desensitisation — was the first behavioural treatment of a specific phobia to be subjected to controlled evaluation. Wolpe's argument drew directly on the conditioning literature: a patient was taught to relax, was asked to imagine the least anxiety-provoking scene on a constructed hierarchy, and was taught to replace the anxiety response with the relaxation response as the hierarchy was ascended. The treatment was radical in its time, because it proposed that a disorder with a fifty-year history of psychological interpretation could be treated by a procedure that took no account of the patient's inner life.

It was also a compromise, and the compromise turned out to matter. Wolpe retained the imagery technique of Jacobson, and the use of imagined scenes rather than actual ones is the element of the treatment that later work identified as its principal weakness: a young person can imagine a scene of the canteen and relax, and can therefore be taught to relax while thinking about the canteen, and can do all of this while still refusing to enter the canteen. The avoidance is intact and the fear is intact. The treatment produced improvement in the laboratory and inconsistent results in the field.

## 1.2 The exposure movement

The 1960s and 1970s saw the gradual replacement of imaginal desensitisation by **in vivo exposure**, in which the feared stimulus is confronted in its actual context. The shift was driven by three developments.

**The recognition of the importance of avoidance.** Clinical observation showed that the patients who improved most were those who did the things they had been avoiding, and that the patients who could imagine their way through a hierarchy without confronting the real stimulus did not maintain their gains.

**The development of behavioural rehearsal and homework**, which moved the confrontation out of the consulting room and into the young person's week. A fear treated only in the consulting room is a fear treated in an unusual context, and it will not generalise.

**The development of the self-administered exposure** in which the young person constructs and executes their own hierarchy, with the therapist as consultant.

## 1.3 The decisive evidence

The comparison of behavioural and psychoanalytic treatment in the 1960s and 1970s produced results that the profession could not ignore, and the results were consistent: the behavioural treatment produced markedly better outcomes on measures of the symptom itself, on measures of the functional impairment, and on the follow-up.

The reason the findings were so decisive is worth understanding, because it is a general methodological lesson. The comparison was made possible by the fact that both treatments could be specified in enough

detail to be delivered consistently, and by the fact that the outcome measures were anchored to the symptom rather than to the process. A treatment that does not relieve the symptom does not work, whatever its theory of change, and the discipline's willingness to measure the symptom rather than the theory was what allowed the evidence to accumulate.

### 1.4 The modern position

Contemporary exposure practice is a synthesis, and the synthesis is more sophisticated than any of its components. The following elements are now standard.

**The behavioural experiment.** The treatment is framed explicitly as an experiment in which the young person is testing a belief, and the therapist's role is to help design the test so that the belief is genuinely put at risk. This framing comes from the cognitive tradition and it has become the dominant way of delivering exposure.

**The alliance and the collaborative protocol.** Modern practice does not impose an exposure on a young person. The hierarchy is constructed collaboratively, the young person determines the pace, and the therapist's task is to make the exposure as safe and as tolerable as the evidence allows while not letting the young person avoid it.

**The modern protocols.** For panic disorder, social anxiety disorder, obsessive-compulsive disorder, and post-traumatic stress disorder, the field now has structured protocols with detailed manuals, and the outcome of a course of exposure therapy is markedly better when a manualised protocol is followed.

**The extension to complex cases.** Exposure therapy is now used routinely with children, with young people with developmental difficulties, and with people whose phobia is embedded in a complicated family or school context, and the adaptations are well described.

## 2. The Core Principles

Five principles run through all of the exposure-based treatments, and a student should be able to state them and explain the rationale for each.

**The feared stimulus must be encountered.** A fear is learned and maintained by the avoidance of the feared stimulus. The prediction that avoidance makes the fear stronger is the basis of the whole treatment, and it is the reason that a young person's refusal to attend school, or to use the canteen, or to go to the dentist, is the target rather than the problem.

**The feared outcome must not occur.** The exposure must be arranged so that the aversive event that maintains the fear does not happen. This is the principle of *inhibitory learning*: the new learning is that the feared outcome does not occur, and the old learning is that it does. The CS–US association is not erased; it is superseded.

**Avoidance must be prevented.** If the young person is allowed to leave the exposure at the first moment of distress, the avoidance is reinforced, the anxiety is relieved, and the fear is strengthened. This is the point at which the treatment becomes a test of the therapist's resolve as well as of the young person's courage, and where the ethical questions in Section 12 are live.

**The exposure must be repeated.** One successful encounter does not constitute learning. Extinction is fragile, and the evidence from the laboratory and the clinic is that a single exposure produces a large but

temporary effect, and that repeated exposures produce a larger and more durable one.

**The young person must do it themselves.** The fear must be confronted in the absence of the safety signals that would otherwise accompany it. This is the point on which the treatments differ most from the classic desensitisation imagery, and it is the point on which modern practice most clearly departs from Wolpe.

Two further points complete the account, and they are refinements rather than principles.

**The learning must be generalised.** The young person's learning should attach to a range of variations, and to the actual contexts in which the fear occurs, not to the therapy room or to the specific therapist.

**The learning must be retrieved.** A single successful exposure is vulnerable to spontaneous recovery, and the treatment must build in retrieval work across time — repeated exposures, in varied contexts, over weeks. The relapse-prevention work of the cognitive tradition and the maintenance phase of the behavioural tradition are the same idea reached from different directions.

# 3. The Varieties of Exposure

## 3.1 In vivo exposure

In vivo exposure is the confrontation with the feared stimulus in its actual physical context, and it is the default for most specific phobias with young people. A young person with a school refusal is taken to school; a young person with a dental phobia is taken to the dentist; a young person with a needle phobia is taken to the clinic.

Its advantages are that it is ecologically valid, that it addresses avoidance in the place where the avoidance occurs, and that it produces generalisation naturally, because the actual context contains all the cues of the actual context and not a reduced set.

Its difficulties are real. It requires planning for every contingency, and it requires the presence of staff who know the plan. It is more taxing on the young person than the alternatives, and it is more taxing on the therapist, because the therapist's anxiety is a legitimate variable in the session and is a reason to prepare carefully rather than a reason to abandon the procedure.

## 3.2 Imaginal exposure

Imaginal exposure uses a carefully constructed set of images, scenes, or narratives, and the treatment is effective in a number of phobias. It is particularly useful where the stimulus is genuinely unavailable — a feared animal, a feared situation that has ended, a feared place that no longer exists — and where the young person can construct vivid images, which is a skill that varies considerably.

Its limitation is the one identified by Wolpe's own successors: the young person can learn to relax while imagining, which is not the same learning as tolerating the real stimulus, and the treatment may therefore produce a change in the imagery response without a change in the avoidance. Where imagery is used, it should be combined with in vivo work, and the young person's own reports of the vividness and the felt response to the images should be monitored.

## 3.3 Interoceptive exposure

Interoceptive exposure deliberately induces the bodily sensations that the young person fears and misinterprets. A young person who believes that a rapid heartbeat means they are about to die is asked to

produce a rapid heartbeat, by jumping on the spot, or by running up and down stairs, and is helped to examine what actually happens.

The technique is useful in panic presentations and in some somatic presentations, and it is one of the simplest and most direct illustrations of the behavioural logic: the feared sensation is produced, the feared outcome does not occur, and the young person learns the difference between the sensation and the catastrophe.

## 3.4 Virtual reality and other mediated exposure

Virtual reality exposure is used where the real stimulus is dangerous, scarce, or prohibitively expensive — fear of flying, of spiders, of heights, of social situations. In child practice it is more often used for social anxiety, where a virtual avatar allows rehearsal of difficult interactions, and for medical procedures, where a virtual environment allows rehearsal of a painful or frightening procedure.

The advantages are ecological control, the ability to repeat without real-world risk, and the ability to generalise across virtual contexts. The limitations are the cost and availability of the equipment, the transfer problem — whether learning in a virtual context generalises to the real one — and the specific difficulty with young people who are frightened by the technology itself.

## 3.5 The exposure hierarchy

Whatever the modality, the treatment requires a **hierarchy**, a ranked list of situations from the most tolerable to the most difficult, each with a subjective rating of the expected distress. Three rules govern its construction.

**The hierarchy is built collaboratively and specifically.** A generic "fear of dogs" hierarchy is useless. The hierarchy is built from the young person's own circumstances: which specific dog, in which specific place, at which specific distance.

**The steps are graded, and the first step is chosen to be successful.** A young person who cannot tolerate the first step of their own hierarchy has been given a hierarchy that is too steep, and the answer is to subdivide.

**The young person rates the distress at each step**, and the ratings are recorded. The rating is both a measure of the progress and the mechanism by which the young person learns that the distress has a shape, rises to a peak, and then falls — which is one of the most useful things a young person can learn about their own fear.

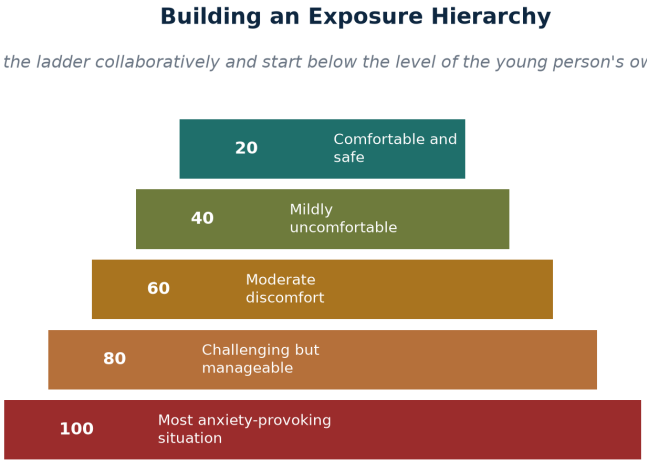



**Figure 25.13** An exposure ladder showing graded steps from a preparatory stage through in vivo confrontation to post-exposure review, with the corresponding distress curve for each stage.

## 3.6 The first-day design

The **first-day** design is the specific contribution of the behavioural tradition to work with young people, and it is the most important developmental adaptation in this chapter.

The design begins with a single, planned, and bounded exposure to the most tolerable version of the feared stimulus, on the first day of treatment, before any substantial assessment, homework, or psychoeducation has been attempted. The young person is asked to do the thing they have been avoiding, in the smallest possible dose, and to report afterwards what happened.

The rationale is developmental and is stated in three claims.

**The young person who has never done the feared thing has no evidence about it.** A fourteen-year-old who has not attended school for four months has no idea what will happen, and the anticipation is invariably worse than the reality. The first-day design supplies the missing evidence.

**A success produces a treatment alliance that no amount of explanation can produce.** The first session often ends with the young person reporting something close to disbelief. That report is the most powerful single event in the early phase of therapy, and it is available only by doing the thing.

**It is diagnostic.** The young person's response to the first-day exposure tells the counsellor more about the maintaining contingencies than an extended assessment would, because it reveals what the young person does when the avoidance is broken.

The design has ethical requirements that must be respected. The exposure must be genuinely small. It must be planned, with the young person's consent to the specific plan and with an explicit understanding of what will happen. There must be a plan for what happens if the young person cannot cope, which means that the counsellor must be willing to end the session, and must be present to end it. And the first-day exposure must not be a bluff, and must not be a test of whether the young person is "ready".

# 4. Rules of Exposure Practice

Modern manualised protocols specify the rules of exposure practice in detail, and the rules are the treatment. The core elements are as follows.

**Repeated, planned exposure to the feared stimulus.** Not a single graded session, but a programme of exposures distributed across the treatment.

**A clearly specified goal for each session**, agreed with the young person. "Stay in the corridor for five minutes and speak to one person" is a goal. "Do some exposure" is not.

**Reminders of the feared outcome.** A surprising number of young people find that the feared outcome does not occur, but that they have never actually checked. The exposure protocol includes a planned check of the feared outcome, and this is one of the most important elements of the modern protocols, because it directly targets the *misappraisal* that maintains the fear.

**Repetition of the successful exposure.** The young person is asked to repeat the successful exposure, often in a slightly harder form, because a single success can be attributed to chance or to the presence of the therapist, and the attribution must be dislodged.

**Prevention of avoidance and safety-seeking.** The young person is asked to give up safety behaviours — the held breath, the reassurance-seeking, the companion, the escape route — and this is often the hardest and most important element.

**Work on the feared outcome and the feared self-image.** The young person is helped to examine what the feared outcome would mean, and what it would mean about them if it occurred, and the cognitive work of the exposure protocol is directed at these two targets.

**Attention to the young person's own predictions.** The young person is asked to predict their distress beforehand and to compare the prediction with what occurred. The comparison of prediction with outcome is the mechanism that produces the learning, and it is why the therapist is a consultant rather than a director.

**Grading the difficulty.** The exposure is graded so that it is difficult but achievable, and the young person's response is used to recalibrate. Repeatedly assigning a task that is far beyond the young person's capacity produces abandonment; repeatedly assigning one that is easy produces no learning.

**Homework between sessions.** The work continues outside the session, and the homework is the point at which the generalisation occurs.

**No pharmacotherapy dependence for the exposure work.** Where medication is used for a co-occurring condition, the protocol is designed so that the exposure is not the thing that the medication is doing.

# 5. Modelling

## 5.1 The principle and the evidence

**Modelling** is the demonstration of a behaviour by another person, with the expectation that the observer will acquire or perform the behaviour. Bandura's social learning theory established three mechanisms by which it works: **attention** to the model, **retention** of the modelled behaviour, and **reproduction** of the behaviour, which requires the physical and cognitive capacity to perform it and the motivation to attempt it.

The demonstrations that established the phenomenon are among the most cited in the psychology literature. Vicarious reinforcement — the increase in the observer's performance of a behaviour when the model is

reinforced for it — was demonstrated with children and aggressive behaviour, with the striking result that the children reproduced the model's aggressive behaviour even when the model's aggression was punished rather than rewarded. The Bobo doll studies established that children reproduce novel aggressive behaviours they have only seen modelled, and that the reproduction is reduced when the model is punished.

The clinical application followed, and the **modelling** and **participant modelling** procedures are now established components of treatment for phobias, social anxiety, assertiveness deficits, and a range of social skills difficulties. In **participant modelling**, the therapist models the feared behaviour and then guides the young person through it, in a graded sequence, with the therapist present and then progressively less present. In **co-participant modelling**, the model is a peer or a member of the group.

## 5.2 Modelling with young people

The adaptation of modelling for young people has some features that are worth stating.

**The model must be credible.** Children are acute judges of whether a model is competent, and a therapist modelling a task they are not good at is worse than useless. Where the therapist is not an appropriate model, a peer, a teacher, or a video is used.

**Multiple models are better than one.** A young person who has seen several different people perform the feared behaviour has more options available to them as a model.

**The modelling must be accompanied by information.** The observed behaviour is more likely to be adopted when the young person understands what is being modelled and what the expectations are.

**The fear must be addressed in the modelling itself.** In the treatment of a specific phobia, the model demonstrates the feared behaviour *while remaining calm*, and the demonstration of the discrepancy between the young person's prediction and the model's actual experience is the therapeutic element.

**Peer modelling has particular value with social anxiety and with school refusal**, because the peer can demonstrate that the feared interaction is survivable in a way that an adult cannot.

## 5.3 Behavioural rehearsal

**Behavioural rehearsal** is the practice of the behaviour in a graded sequence, in the presence of supportive feedback, before the young person attempts it in the natural setting. It is the component that makes modelling into treatment, because it is the transfer of the demonstrated behaviour into the young person's own repertoire.

Rehearsal has three components that should be distinguished.

**Instruction** — the explicit teaching of the behaviour, including a rationale and a script. This is the component that most clearly joins the behavioural and the cognitive traditions.

**Rehearsal** — the performance of the behaviour in a low-risk setting, with feedback.

**Homework** — the performance in the natural setting, which is the generalisation step.

The distinction matters because the three components fail differently. A young person can have a perfect rationale and fail at rehearsal; a young person can rehearse perfectly and fail at homework; and a young person can succeed at homework and not maintain it, if the homework is not repeated.

## 5.4 Cognitive behavioural rehearsal

The modern version adds the cognitive component explicitly: the young person rehearses not only the behaviour but the *belief* that the behaviour is built on, and the sequence is deliberately run in the "worst case" form. A young person who believes that any question from a teacher means they are in trouble rehearses a range of teacher questions, including hostile ones, and the point is that the rehearsed scenarios are survivable.

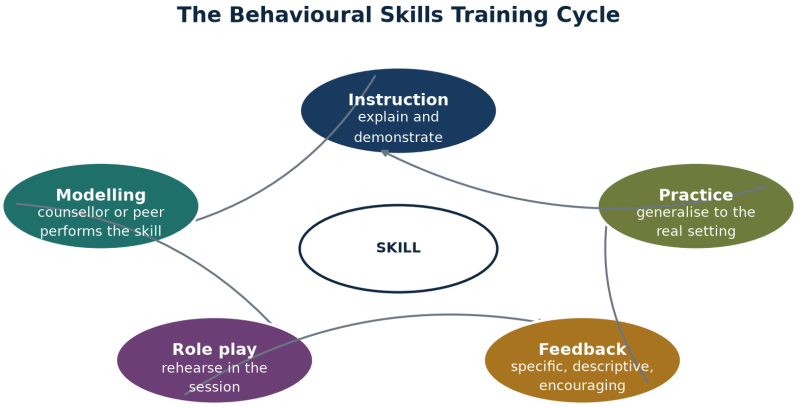



**Figure 25.14** A behavioural skills training cycle showing instruction, rehearsal, feedback, and homework, with the bidirectional arrows connecting the components to the target skill and to the young person's confidence.

# 6. Behavioural Skills Training

## 6.1 The four components

**Behavioural skills training** is a structured approach to the teaching of a specific interpersonal or self-management skill, and it has four components, which are usually taught in a fixed order.

1. **Instruction** — an explicit description of the target skill, in concrete behavioural terms, with a rationale, and often with a model of the behaviour. The instruction is specific: not "be more assertive" but "when your mother says you're going to bed at nine, say 'I've still got homework' in a calm voice and don't raise it to a shout."
2. **Modelling** — a demonstration of the skill, by the therapist, by a peer, or on video. The model demonstrates both the behaviour and the calmness, which is the point.
3. **Rehearsal** — the young person's own performance of the skill, with coaching and feedback, repeated until the performance is adequate.
4. **Homework** — the performance of the skill in the natural setting, between sessions, with the recording of attempts and outcomes.

The order is not arbitrary. Instruction without rehearsal produces a young person who can describe the skill and cannot perform it; rehearsal without homework produces a skill that exists only in the room.

## 6.2 The skills that are taught

The most common targets in work with young people are: **basic interpersonal skills** (conversation initiation, asking for help, saying no, disagreeing, apologising); **assertiveness**; **anger-management skills**, including the recognition of the early physiological signs of arousal and the use of a response delay; **problem-solving**; **social skills** including the interpretation of others' nonverbal cues; **emotion-regulation skills**; and **study and organisational skills**.

The selection of the target skill should be a clinical decision, and the criteria are straightforward: the skill should be **specific** enough to be taught and observed; it should be **functional** in the young person's life; it should be **attainable** in the developmental context; and the young person should have a reason for acquiring it. A skill that the young person does not want is not acquired.

### 6.3 The relationship to other traditions

Behavioural skills training is where the behavioural and cognitive traditions are most completely integrated, and a student should be able to trace the components. The **instruction** component is a cognitive intervention, in that it works on the young person's attributions and expectations about the skill. The **modelling** component is social learning theory. The **rehearsal** component is behavioural rehearsal with in-session feedback, and its efficacy depends on the behavioural principle that behaviour is selected by its consequences, which is why the feedback must be immediate and specific. The **homework** component is generalisation training.

What behavioural skills training does not do is deal with the underlying motivation, the underlying relationship, or the underlying developmental context. A young person who is taught assertiveness and who lives in a home where assertiveness is punished has acquired a technique that will be extinguished, and the practitioner who stops at the skills training has not finished the work.

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## 8. Exposure with Children and Adolescents

The adaptations for young people are not cosmetic. Each of them follows from a developmental fact, and a student should be able to state the fact and the adaptation.

### 8.1 Cognitive capacity and the prediction–outcome comparison

The central mechanism of contemporary exposure is the comparison of the young person's prediction with the observed outcome, and this mechanism requires a capacity to form a prediction, to hold it, and to compare. That capacity is still developing in a twelve-year-old and is weaker still in a young person with a learning difficulty or an intellectual disability.

The adaptations follow directly. The prediction is made explicit and written down, or drawn, so that the comparison is concrete rather than retrospective. The comparison is made immediately after the exposure rather than at the next session. The scale for rating distress is a visual analogue scale or a numbered scale rather than a verbal report. And with young people whose capacity for prediction is limited, the emphasis shifts further towards the behavioural components — the modelling, the graded practice, the repetition — which do not require the prediction at all.

### 8.2 The developmental gradient of the fear

Young people's fears are not distributed evenly across the developmental range, and the form the fear takes predicts the treatment.

**Early childhood.** Fears are concrete, specific, and largely realistic: monsters, the dark, thunder, doctors. Treatment is largely parental and involves graded exposure by the parent with the therapist as consultant, and it works well.

**Middle childhood.** Fears become more elaborate and more social. School phobia, often with somatic presentation, is a characteristic middle-childhood presentation, and the treatment involves a graded return to school with a specific first-day design, coordination with the school, and often a plan for the parents, whose anxiety is frequently the barrier.

**Adolescence.** Fears become more abstract and are more often embedded in identity: performance fears, social evaluation fears, fears about the future. The cognitive component of the exposure becomes more prominent, and the work on the feared self-image becomes more important, because at this age the feared outcome is frequently not a stimulus but an identity — being thought to be weak, thought to be a failure, thought to be nothing.

## 8.3 The developmental gradient of the control of the situation

A young person's control over the exposure varies enormously, and the treatment must be built around the actual degree of control.

**A very young child has no control.** The parent or the teacher controls the exposure entirely, and the therapist's role is to coach the adult. This is a common and underestimated form of behavioural work with young people, and the skill required is coaching adults rather than treating children.

**A mid-child has partial control.** The young person can choose the pace within a structure that others set, and a good treatment uses this: the adult establishes the non-negotiable elements, the young person controls the rest.

**An adolescent has substantial control, and the treatment should reflect it.** The exposure is designed collaboratively, the young person sets the goals, and the therapist's role is explicitly consultative. An exposure imposed on an adolescent is likely to fail for reasons that have nothing to do with the theory.

## 8.4 The role of the parent

The parent's role requires explicit thought, and the literature is clear that the parent's anxiety is one of the strongest predictors of poor outcome in the exposure treatment of children's anxiety.

Three practical rules follow. First, the parent is coached before the exposure begins, on what to say, what not to say, and how to respond to the young person's distress, and the coaching is specific. Second, the parent's own distress is addressed directly and compassionately, because a parent who is frightened of the school will inadvertently signal that the school is dangerous, and the young person will learn to read that signal. Third, the parent is given a role that is useful rather than a role that is anxious, which usually means a specific practical job in the exposure rather than a watching brief.

There is an important caution about the parental accommodation literature. Sustained parental accommodation of a child's anxiety is now understood to be a maintaining factor rather than a supportive one, and parents frequently need to be helped to stop accommodating. This is a hard message to deliver, and it should be delivered with the recognition that the parent has been doing the accommodating out of love, and with a great deal of support.

## 8.5 School refusal

School refusal is one of the most common referrals in school counselling and one of the most heterogeneous, and the behavioural framework requires that it not be treated as a single condition.

A functional analysis of the school refusal is the first step, and it distinguishes at least four patterns: refusal maintained by **escape** from a specific aversive school situation, often a bullying situation or a particular class; refusal maintained by **anxiety** about a specific social or performance demand; refusal maintained by **attention**, in which the attendance pattern has become a negotiation with the parents; and refusal

maintained by **somatic or medical symptoms**, in which the child is genuinely unwell and the school is the setting in which the illness is expressed.

Each of these has a different treatment, and the commonest clinical error is to treat all of them with the same graded return to school, which works for the escape-maintained case and fails for the others.

**The first-day design for school refusal** has a specific and well-described form: the return to a very brief, highly structured, and rewarding visit, often on a day when the feared class is not timetabled, and with a specific activity, a specific adult, and a specific reward. The young person returns to the building, does one specific thing, and leaves having had a good experience. The second visit is longer. The plan is written down and shared with the school, and the school must be ready to receive the visit.

### 8.6 Social anxiety and the school environment

Social anxiety in the school setting requires a plan that extends beyond the therapy room, because the feared situations occur in corridors, at break, in the dining hall, and in the classroom, and the treatment must involve the whole school.

The components of such a plan typically include: a designated safe adult; a pre-arranged escape route that the young person can use without it being a visible event; a rehearsal of the specific interactions with the peers concerned; a graded ladder of classroom participation; and, in many cases, a group-based social skills intervention, which is more effective than individual work for social anxiety and has the additional benefit of normalising the difficulty.

### 8.7 Adaptive and maladaptive response styles

Young people vary in how they respond to exposure, and three response styles are worth distinguishing.

**The healthy response** is fear that rises, peaks, and falls, with the young person remaining in the situation and observing the decline.

**The avoidance response** is the young person who is distressed, who then asks to leave, and who is allowed to leave. The escape is the treatment failure, and it is the most common one.

**The agitated response** is the young person who is distressed and who becomes increasingly agitated, aggressive, or distressed to a degree that the session cannot continue. This response requires a different approach, usually involving a lower first-day dose, a greater emphasis on modelling, and a slower grade.

A student who can distinguish these three responses in a session has acquired a clinically important skill.

## 9. Case Illustration: The Young Person Who Would Not Take the Lift

**Presentation.** Tomas is fourteen. He has not used the lift at his school for two years, and the consequence is that he arrives late every day, misses the first lesson, and is spoken to by the pastoral team. His mother reports that he will use the stairs at home, including in a six-storey building, but refuses the lift "because of the people."

The referral came from the school, which describes the problem as defiance. The first session with Tomas establishes that it is not.

**The assessment.** Two questions are asked before anything else is planned.

*What happens in the lift?* Tomas says that the lift is small, that there are usually other students in it, and that he is sure someone will notice him and say something. He describes in detail a sequence in which a group of boys from another year enter the lift, look at him, and start laughing. When asked how often this happens, he says "every time."

*What happened the first time?* This question takes some time, and the answer that emerges is that in his first year at the school, a group of students had been in the lift and one of them had said something about his shoes. He has never re-entered the lift alone since.

The anxiety and the avoidance are now clear, and the functional hypothesis is stated explicitly: **the behaviour — using the stairs — is maintained by escape from the feared social situation, and the escape is reinforced by the absence of the feared outcome and by the relief of the anxiety, and the young person's belief that the situation is genuinely dangerous is maintained by the absence of any disconfirming evidence, which the avoidance guarantees.**

**The plan.** The plan is built around the fact that the feared outcome is social, that the feared situation involves a small enclosed space with an uncontrollable group of peers, and that the young person has no evidence about what actually happens.

The first-day exposure is designed to be small and to be a success.

*Preparation.* Tomas is shown the lift interior in the school reception area, with a staff member standing beside it, and he walks past it twice. That is the whole of the first day. It is small enough that it is almost trivial, and it is selected because it will succeed.

*The second exposure.* On the following day, Tomas enters the lift with the pastoral support worker, goes down one floor, and exits. The counsellor has agreed in advance what will be said if anyone laughs, and has established with the head of year that there is a specific plan for the handling of any incident, because a plan that depends on nothing happening is not a plan.

*The third and fourth exposures.* Tomas enters the lift with a peer he chooses — a friend who does not make him more anxious — and stays for two floors. The friend's presence is a safety signal in the early phase and the purpose is to establish that the situation is survivable with another person present, and the plan to build toward the unaccompanied exposure is explicit from the start.

*The unaccompanied exposure.* By the second week, Tomas enters the lift alone, and the primary and secondary work of the treatment begins: the "checking" of the feared outcome, in which Tomas is asked after each exposure what actually happened, and the two behaviours he had never done, which are the ones that the avoidance was protecting him from. The first time he rides the lift with a group of boys in it and nothing happens, the session is over.

**The cognitive work.** The exposure is not only behavioural. Two beliefs are examined explicitly, because the exposure gives the young person the opportunity to disconfirm them and the therapist's job is to help him notice that he has.

*The catastrophic belief.* "Everyone will laugh." The check: after each exposure, what actually happened? The accumulated evidence is that in eleven exposures, one boy made a comment about a game he was playing on his phone. Tomas is asked to rate how much that comment had in common with his prediction. The discrepancy is enormous, and it is the discrepancy that constitutes the learning.

*The self-image belief.* "I am the kind of person who gets laughed at." This is the deeper target and the one that is not resolved by a single successful exposure. The work is on the difference between the old identity

— which was constructed in a specific incident in a specific year — and the young person's current account of himself, and it is done collaboratively, in the manner of the narrative work of Chapter 36.

**The school plan.** Tomas still arrives late, because the school day starts with a first lesson and he cannot realistically be at school before it. The school plan addresses this directly: the pastoral team arranges for him to have a quiet breakfast in the room of the form tutor before the first lesson, with the tutor available but not requiring conversation. The aim is to remove the punishing consequence of the lateness, which had been adding a substantial daily burden to the anxiety, and to prevent the avoidance from being reinforced by a second mechanism — the relief of not being noticed.

**The outcome and the interpretation.** Tomas uses the lift alone within three weeks, with an occasional deliberate re-exposure, and the work continues on the social evaluation belief for a further two months. His lateness continues and the school has agreed not to pursue it, which was a decision that required the counsellor to make the case for, and which was correct: the pursuit of the punctuality target would have provided a strong reinforcement for the avoidance.

Two features of this case generalise. The first is that the referral question ("defiance") and the presenting problem (avoidance) had nothing to do with each other, and that the functional assessment is what separated them. The second is that the exposure alone would not have been sufficient, because the young person needed a specific plan for the school environment, and because the plan for the lateness was as important as the plan for the lift. Exposure works within a context, and the context is usually doing as much work as the exposure.

# 10. Evidence and Research Support

## 10.1 The status of the evidence

The evidence for exposure-based treatments is among the strongest in the psychological literature, and it is strongest where the theory would predict it to be strongest: for specific, well-defined fears, with a clear functional relation between the stimulus and the fear, and with the avoidance behaviour as the target.

The key findings are as follows.

**Exposure is the best-supported treatment for specific phobias.** The evidence base is large, the effect sizes are large, and the treatment is brief by any standard.

**Exposure-based treatments are effective for panic disorder, social anxiety disorder, and obsessive-compulsive disorder**, and they are the first-line treatment for all three in most international guidelines, with the protocols specified in detail.

**Prolonged exposure is effective for post-traumatic stress disorder** in adults, with more variable evidence in children and adolescents, and the developmental adaptations are described in Chapter 39.

**The efficacy of exposure does not depend on the young person's insight.** The single most important finding in the comparative research is that psychoanalytic interpretation and exposure produce different results, with exposure markedly superior on the outcome measures that matter, and the finding has been replicated.

## 10.2 The evidence with children and adolescents

The evidence with young people is substantial and has grown considerably.

**Individual, clinician-guided exposure** has a large literature across the anxiety disorders of childhood, with meta-analyses reporting large effect sizes for the specific phobias, for social anxiety disorder, for generalised anxiety, and for separation anxiety. The age range extends down to young children in the case of the specific phobias, with parental involvement.

**Parent-delivered exposure** has good support in the specific phobias of childhood and has the advantage of generalising into the family environment, where the feared stimulus usually lives.

**School-based exposure programmes** have good support in school refusal and in social anxiety, and the group formats that include a teacher or a peer are more effective than those that do not.

**Intensive, residential, or clinic-based programmes** for severe school refusal have good support and are an important part of the service landscape, though they are resource-intensive and their availability varies.

**The treatment of OCD in children and adolescents** with exposure and response prevention has a strong evidence base, and the developmental modifications — which are considerable, and which are described in Chapter 25's companion material and in the OCD sections of Part VI — are well specified.

## 10.3 The component studies

The treatment is not a monolith, and the component literature is more instructive than the outcome literature.

**Repeated exposure beats single exposure**, which is the most firmly established component finding, and it is the foundation of the relapse-prevention work.

**Homework between sessions improves outcome**, and the finding holds across anxiety disorders and across age groups. The mechanism is generalisation.

**Predicting the outcome and checking it improves outcome**, and this is the finding that most clearly identifies the modern protocol as different from the classical desensitisation.

**Attention to the young person's predictions improves outcome**, and the studies show that the prediction–outcome discrepancy is the mechanism.

**Working with the therapist present is better in the early phase and worse in the later phase**, and the finding supports the standard clinical progression.

**In vivo is superior to imagery** in the specific phobias, and the finding is now unambiguous.

## 10.4 Limitations of the evidence

Four limitations should be stated honestly.

**The literature is dominated by individual, outpatient, motivated families.** The trials that generate the evidence are largely conducted in well-resourced settings with motivated participants, and the generalisation to a school caseload with reluctant families and logistical constraints is a genuine question.

**The follow-up periods are often short**, and the clinical observation — that the gains are maintained but that a proportion of young people relapse, and that a booster session is effective — is a clinical finding that the trials do not always capture.

**The measures are largely self-report**, which is a particular problem with children, whose reports of their own distress are unreliable in the direction that flatters the treatment, and the field's response has been the

use of behavioural observation measures, of which the school-based avoidance measure developed by clinicians and researchers in this area is the most widely used.

**There is a subgroup for whom exposure does not work well**, including young people with significant developmental disability, young people whose anxiety is a secondary phenomenon of an untreated trauma or of a family system that is continuing to be unsafe, and young people with high levels of parental accommodation. These are not failures of the treatment; they are indications that the assessment was incomplete.

# 11. Strengths and Limitations of Exposure-Based Work

## 11.1 Strengths

**It is the best-evidenced treatment in the field.** For specific phobias and for several other anxiety disorders, exposure has more evidence behind it than any other intervention, and this is a statement about the quality of the research as well as about the effectiveness of the method.

**It is brief and economical.** Many young people are treated in a small number of sessions, which matters greatly in services with long waiting lists.

**It is portable.** The principles transfer to any setting, and the method requires no equipment, no special room, and no specialist apparatus.

**It is compatible with other approaches.** A young person can receive exposure within a cognitive therapy, a behavioural therapy, a family therapy, or a school-based service, and the exposure component is identifiable and specific within it.

**It gives the young person something.** A successful exposure produces a piece of evidence, and the young person keeps it. The therapeutic alliance is often established in the first session by the treatment itself.

## 11.2 Limitations

**The early sessions are unpleasant.** The treatment works by producing distress, and a young person who is asked to endure distress deserves a practitioner who is honest about the fact and who is prepared to sit with them while it happens.

**The therapeutic relationship can be damaged by the treatment.** There is a period in every course of exposure in which the young person experiences the therapist as the person who is making them do the thing they most fear. The alliance usually recovers, and it recovers faster where the therapist has prepared the young person for that reaction in advance.

**It is easily misapplied.** A treatment that involves deliberately provoking fear can be used in ways that damage the young person, and the field's safeguards are designed to prevent exactly that. The recent history of poorly controlled internet-delivered exposure with adults is a warning, and with young people the stakes are higher.

**It does not address everything.** Exposure treats the fear. It does not repair a family system, undo a trauma, treat a depression, or resolve a conflict about authority. Chapter 33 and Chapter 38 address the cases in which the fear is embedded in a system that must also be addressed.

**The young person's motivation is a precondition, and it is not always present.** A young person who has no interest in overcoming the fear, or who is in an environment where overcoming it would be dangerous, is not a suitable candidate for the treatment as delivered, and the assessment of motivation is part of the assessment rather than an afterthought.

# 12. Cultural and Ethical Considerations

## 12.1 Culture, fear, and the meaning of avoidance

The question of what a young person is afraid of, and what the fear means, is culturally patterned in ways that the exposure model must not overwrite.

**Some fears are collectively held and socially sanctioned.** A young person who is afraid of the dark, of particular foods, of particular religious or cultural practices, or of particular social obligations, may be reflecting a shared and respected understanding of the world rather than an individual pathology. A treatment that extinguishes such a fear without understanding its social context removes the young person's connection to a community, and the young person will be supported in maintaining the fear by everyone around them.

**Some fears are real.** A young person who is afraid of a particular adult, of a particular neighbourhood, of a particular school corridor, or of a particular person in the family, is making a reasonable assessment of a real danger, and an exposure programme that confronts that stimulus without a safety assessment is a serious ethical failure. Every exposure plan requires an explicit check of whether the feared outcome is possible, and the discipline's protocols are explicit that exposure is contraindicated where the threat is real.

**Avoidance can be a cultural value.** Familial and community norms about modesty, about respect for elders, about appropriate interaction, and about the visibility of distress all affect what a young person will do in a waiting room, in a classroom, in a shop, and in a group of peers. The exposure plan has to be compatible with the young person's cultural context or it will be undone by it.

**The culture of the school system matters.** A school in which public behaviour is highly visible and where a young person's distress is noticed by everyone will present a different exposure problem from a school in which it is not.

## 12.2 The ethics of deliberately producing distress

The ethical analysis of exposure is unusually rich, and a student should be able to argue it.

**The core claim.** Deliberately inducing distress in a young person in order to remove distress is defensible only when four conditions are met: the disturbance is not needed to live a full life (that is, the fear is genuinely impairing); the person has consented to the specific procedure, with the capacity to withdraw; the intervention is expected to work, on the evidence, and the professional is competent to deliver it; and the distress is not being produced for the convenience of the practitioner or the institution.

**The consent condition.** Consent to a treatment in general is not consent to a specific exposure, and the consent must be renewed as the treatment proceeds, because the nature of the exposure changes as the hierarchy ascends. A young person who consented to sitting in the corridor may not have consented to a group of peers being assembled in front of them, and the difference matters.

**The capacity to withdraw.** A young person must be able to stop the exposure, and must be able to do so without the withdrawal being treated as a failure of treatment or as evidence about them. A practitioner who takes personally a young person's request to stop will find that they get more, not fewer, completed exposures, because the young person knows that the option is real.

**The proportionality condition.** The intensity of the exposure must be proportionate to the impairment. A young person with a mild school anxiety and a young person unable to leave the house are not receiving the same treatment, and the second case usually requires a more careful, more gradual, and more supported approach than the first.

**The non-maleficence condition.** The exposure must not produce harm. Where the feared outcome is possible, where the young person has a vulnerability that the exposure would activate, or where the exposure would exceed the young person's developmental capacity, the procedure must be modified or abandoned.

**The right to a second opinion.** A young person, or a family, who has serious concerns about an exposure programme should be able to obtain one, and the practitioner should be able to name a colleague who could provide it.

### 12.3 Specific ethical considerations with young people

**The parent's role in the decision.** Where the parents are the ones who are refusing the treatment on the young person's behalf, the ethical question is usually not about the parent but about what is at stake for them, and it is worth exploring what the parent's own fear or expectation is.

**Withdrawal in a school setting.** A young person who withdraws from an exposure in school needs the withdrawal to be handled in a way that does not create a public event, and the school needs a written plan for this in advance, because it will happen.

**Cultural and religious considerations.** Where the young person's family has a practice that the exposure programme would disrupt — a diet, a fasting period, a practice of avoidance that has religious significance, a gender-related expectation about contact with opposite-sex people — the programme must be designed around it. This is a practical matter as much as an ethical one, and the practitioner should ask.

**Documentation.** The exposure hierarchy, the young person's consent, the agreed stopping points, and the review points should be documented, because the programme is unusual enough that a careful record is a professional necessity, and because a record is the young person's protection as much as the practitioner's.

## 13. Key Terms

**Key Terms at a Glance**

- **Exposure** — The planned, systematic presentation of a feared stimulus, in conditions in which the feared outcome does not occur, for the purpose of producing new learning about the relationship between the stimulus and the outcome.
- **In vivo exposure** — Exposure conducted in the actual physical context in which the fear occurs; the default modality for most specific phobias with young people.
- **Imaginal exposure** — Exposure using constructed images, scenes, or narratives, used where the real stimulus is unavailable.
- **Interoceptive exposure** — The deliberate induction of feared bodily sensations, used with panic and somatic presentations.
- **Virtual reality exposure** — Exposure conducted in an immersive virtual environment, used where the real stimulus is dangerous, scarce, or expensive.
- **Exposure hierarchy** — A collaboratively constructed, individually specific, and graded list of feared situations with subjective distress ratings, ordered from most to least tolerable.
- **First-day design** — The practice of beginning treatment with a single, small, planned, and bounded exposure to the feared stimulus, chosen to succeed, before substantial assessment or homework.
- **The law of effect applied to fear** — The principle that avoidance is negatively reinforced by the relief it produces, and that the avoidance therefore maintains the fear.
- **Inhibitory learning** — The account of exposure as the learning that the feared outcome does not occur, which supersedes rather than erases the original learning.
- **CS–US association** — The learned association between the conditioned stimulus and the aversive unconditioned stimulus, the interruption of which is the aim of exposure.
- **Safety signal** — A stimulus that reliably accompanies the feared situation and thereby signals safety; its use is helpful in the early phase and must be faded.
- **Safety-seeking behaviour** — Behaviour that reduces the perceived danger at the cost of maintaining the fear; a primary target of exposure.
- **Avoidance** — The behaviour of escaping or postponing a feared situation, negatively reinforced by the relief produced; the principal maintaining factor in anxiety disorders.
- **Behavioural experiment** — A collaborative test of a specific prediction, designed so that the belief is genuinely at risk of disconfirmation.
- **Prediction–outcome discrepancy** — The comparison of the young person's expected outcome with the observed outcome; the mechanism by which the learning occurs.

## Key Terms at a Glance (continued)

- **Exposure protocol** — A manualised specification of the elements and rules of an exposure treatment, with defined sessions, homework, and review points.
- **Prolonged exposure** — A structured protocol for the treatment of post-traumatic stress disorder, with a focus on the emotional processing of the traumatic memory.
- **Exposure and response prevention** — The behavioural treatment of obsessive-compulsive disorder, combining exposure to the obsession with the prevention of the compulsion.
- **Grade of exposure** — The degree of difficulty assigned to a particular exposure, calibrated so that it is challenging but achievable.
- **Retrieval work** — Repeated re-exposure across time and contexts, designed to protect the learning against spontaneous recovery and relapse.
- **Modelling** — The demonstration of a behaviour by another person, through which the observer acquires or performs the behaviour.
- **Participant modelling** — A graded procedure in which the therapist models the feared behaviour and then guides the young person through it with progressively less assistance.
- **Co-participant modelling** — Modelling in which the model is a peer or a group member rather than the therapist.
- **Vicarious reinforcement** — The increase in the observer's performance of a behaviour following the observation of a model being reinforced for it.
- **Observational learning** — Bandura's account of learning through attention, retention, and reproduction, without direct reinforcement of the observer.
- **Behavioural rehearsal** — The graded practice of a behaviour in a low-risk setting, with feedback, before performance in the natural setting.
- **Instruction** — The explicit teaching of a target behaviour, its rationale, and often a script; the first of the four components of skills training.
- **Behavioural skills training** — A structured four-component method for teaching a specific interpersonal or self-management skill: instruction, modelling, rehearsal, and homework.
- **Homework** — The planned practice of a target behaviour in the natural setting between sessions; the generalisation step of skills training and of exposure.
- **Target behaviour** — The specific, observable behaviour that a treatment is intended to produce, defined in operationally countable terms.
- **Avoidance of the exposure** — The premature termination of an exposure, which is the commonest form of treatment failure and which is reinforced by the relief it produces.

**Key Terms at a Glance (continued)**

- **Agitated response** — A pattern in which distress escalates to a degree that the session cannot continue, requiring a lower dose, more modelling, and a slower grade.
- **Parent-delivered exposure** — Exposure in which the parent is coached to conduct the hierarchy, which generalises the learning into the family environment.
- **Parental accommodation** — Sustained parental modification of family routine and rules in response to the child's anxiety, which is now understood as a maintaining factor.
- **School refusal** — A pattern of persistent non-attendance at school that requires functional analysis before treatment, because at least four distinct maintaining patterns are recognised.
- **Relapse prevention** — The set of procedures designed to maintain gains after the end of treatment, including booster sessions, retrieval work, and written plans.
- **Booster session** — A planned follow-up session after the end of treatment, offered to consolidate gains and to address early signs of return.
- **Proportionality** — The ethical requirement that the intensity of an intervention be proportionate to the severity of the impairment it addresses.
- **Informed assent** — The young person's own agreement to a specific procedure, given with the capacity to decline and to withdraw.

## 14. Chapter Summary

Exposure is the most successful behavioural intervention in the history of the profession, and it emerged from a theoretical dispute whose resolution was methodological as much as theoretical. The psychoanalytic account held that a phobia was a symbol; the learning account held that it was an acquired and maintained fear. The comparative evidence showed that the treatment that followed from the learning account worked and the treatment that followed from the symbolic account did not, and the field took from this the lesson that a treatment which does not relieve the symptom does not work, whatever its theory.

The five principles of exposure were stated in this chapter: the feared stimulus must be encountered; the feared outcome must not occur; avoidance must be prevented; the exposure must be repeated; and the young person must do it themselves. Each was examined in relation to the developmental context, and the chapter developed the specific adaptations that follow from the cognitive capacity of the young person, the developmental gradient of the fear, the gradient of the young person's control over the exposure, the role of the parent, and the school context.

The varieties of exposure — in vivo, imaginal, interoceptive, and virtual — were described with their indications and limitations, and the hierarchy and the first-day design were set out in detail, because the details are what make the treatment safe. The rules of exposure practice were specified, and the single most important of them is the prevention of avoidance, since the premature termination of an exposure is the commonest treatment failure and is itself reinforced.

Modelling was described through Bandura's mechanisms of attention, retention, and reproduction, and its adaptation for young people was set out with the emphasis on credibility, multiple models, and the

demonstration of discrepancy between prediction and outcome. Behavioural rehearsal and behavioural skills training were described in their four components, and the chapter traced the components to their theoretical origins, making explicit that skills training is where the behavioural and the cognitive traditions are most completely integrated and that it does not deal with motivation, relationship, or developmental context.

The case of Tomas illustrated the separation of a referral question about defiance from a presenting problem of avoidance, the construction of a graded exposure with a planned check of the feared outcome, the work on the self-image belief, and the necessity of a school plan that included a decision not to pursue the punctuality target, since its pursuit would have provided a strong reinforcement for the avoidance.

The evidence is among the strongest in the literature for the specific phobias, for panic disorder, for social anxiety disorder, and for obsessive-compulsive disorder, and is substantial and growing for the anxiety disorders of childhood and adolescence. The component studies are more instructive than the outcome studies, and they identify repeated exposure, homework, the prediction–outcome check, and attention to predictions as the elements that make the difference.

The ethical analysis is the richest in the book, because the treatment deliberately produces distress. The conditions of defensibility are the necessity of the distress, the consent to the specific procedure, the expectation of benefit, and the absence of institutional convenience. The cultural analysis adds the point that some fears are collectively held and some fears are real, and that an exposure programme that confronts a real danger without a safety assessment is a serious ethical failure.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the theoretical dispute between the psychoanalytic and the learning accounts of phobia, and state how it was resolved.
2. State the five principles of exposure and explain the rationale for each.
3. Distinguish in vivo, imaginal, interoceptive, and virtual exposure, and state the indication for each.
4. Explain the three rules governing the construction of an exposure hierarchy.
5. Describe the first-day design and state the three claims in its rationale.
6. State the ethical requirements that must accompany a first-day exposure.
7. State the core rules of exposure practice and explain the significance of each.
8. Explain what is meant by an inhibitory account of the learning in exposure.
9. Describe the "checking" of the feared outcome and explain why it is central to the modern protocol.
10. Describe the prevention of safety-seeking behaviour and explain why it is difficult.
11. Explain Bandura's three mechanisms of modelling, and describe participant modelling and co-participant modelling.
12. Describe the four components of behavioural skills training and explain why the order is fixed.
13. Explain how behavioural skills training integrates the behavioural and the cognitive traditions.
14. State the four adaptations of exposure that follow from the developmental status of the young person.
15. Explain the significance of the parent's anxiety in the treatment of a young person's anxiety.
16. State what is now understood about parental accommodation and what a practitioner should do about it.
17. Describe the four maintaining patterns in school refusal, and explain why a single treatment plan fails.
18. Describe the first-day design as applied to school refusal.
19. Distinguish the healthy response, the avoidance response, and the agitated response in an exposure session.
20. Describe the case of Tomas and explain the two features of it that generalise.
21. Summarise the status of the evidence for exposure with specific phobias, panic disorder, social anxiety disorder, and obsessive-compulsive disorder.
22. Summarise the component findings about repeated exposure, homework, checking the feared outcome, and the prediction–outcome comparison.

**Review and Discussion Questions (continued)**

23. State the four limitations of the exposure evidence base.
24. State the four conditions under which deliberately inducing distress in a young person is defensible.
25. Explain the two cultural considerations that may make an exposure plan inappropriate or require its modification.

## 16. Practice Activities

**Practice and Application**

### Activity 25.1 — Building a First-Day Exposure (75 minutes, individual)

Choose a young person you know or a case from placement who is avoiding something. Build the smallest possible first-day exposure: what it is, how long it lasts, who is present, what is said beforehand, what the young person is told they will feel, what the stopping point is, and what the young person does immediately afterwards. Then write the sentence you would say to the young person to introduce it. The purpose is to discover how hard it is to make the exposure small enough, and most students find that their first design is much larger than the one they end up writing.

### Activity 25.2 — The Prediction–Outcome Table (60 minutes, individual)

Take a fear of your own — a genuine one, however small — and complete ten rows of a table with the columns: situation, predicted outcome, predicted distress (0–100), actual outcome, actual distress, and the discrepancy. Do not complete the table from memory at the end; fill in each row immediately after the event. Then write what you would now believe if you were to design a similar table for a young person. The exercise demonstrates the mechanism, and it also demonstrates how much of the anticipatory dread is produced by the absence of a record.

### Activity 25.3 — Skills Training Rehearsal (90 minutes, small group, three roles)

In groups of three, one student plays a fourteen-year-old who is afraid of raising a problem in front of a teacher, one plays the teacher, and one plays the skills trainer. The trainer must run the four components — instruction, modelling, rehearsal, homework — on the target behaviour of asking a question in a lesson. Then swap roles so that each student plays the young person. The role of the young person is the instructive one, because it exposes the difference between a behaviour that has been rehearsed and a behaviour that has been modelled, and the difference between a young person who is coached and a young person who is instructed.

### Activity 25.4 — The Refusal and the Withdrawal (60 minutes, pairs, discussion)

In pairs, one student plays a parent who is convinced that their thirteen-year-old's refusal to attend school is a deliberate act of defiance and that the treatment should involve consequences, and the other plays the counsellor. The counsellor's task is to establish the maintaining contingencies without dismissing the parent's account, and to explain the parental accommodation literature without implying that the parent has been doing the wrong thing. Then reverse roles. The exercise is the best preparation in the book for the first family meeting in a school refusal case.

## 17. Annotated Reading Pathways

**For the treatment.** Read Wolpe (1958) for the origin and Bennett-Levy and colleagues (2016) for the modern protocol. The distance between the two is the history of the field in one comparison.

**For the evidence.** Read the meta-analyses of exposure with children and adolescents and read the component reviews on homework and on the prediction–outcome check. The component reviews are more useful clinically than the outcome reviews, because they tell you which parts of the protocol to protect when time is short.

**For the child-specific adaptations.** Read O'Connell (2009) in full. It is the most practically usable of the child-focused exposure texts and it addresses the first-day design, the school-based work, and the parental

role in a form that can be used directly.

**For the case conceptualisation.** Read the chapter on anxiety disorders in the child-focused clinical texts and read the school refusal literature, which contains the most useful functional analyses available in the applied literature.

**For the ethics.** Read the ethical guidelines for the behavioural approaches and the critical literature on exposure with minors, and read the material on parental accommodation, which has implications well beyond exposure therapy.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Exposure, Modeling, and Behavioral Skills Training** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the treatment of avoidance, fear, and deficit through systematic desensitisation, observation, and rehearsal. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or

school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Amara is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Amara's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Amara explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Amara chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Amara has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is

redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Amara can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?

- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Exposure, Modeling, and Behavioral Skills Training

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Exposure, Modeling, and Behavioral Skills Training** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Exposure, Modeling, and Behavioral Skills Training** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider

disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

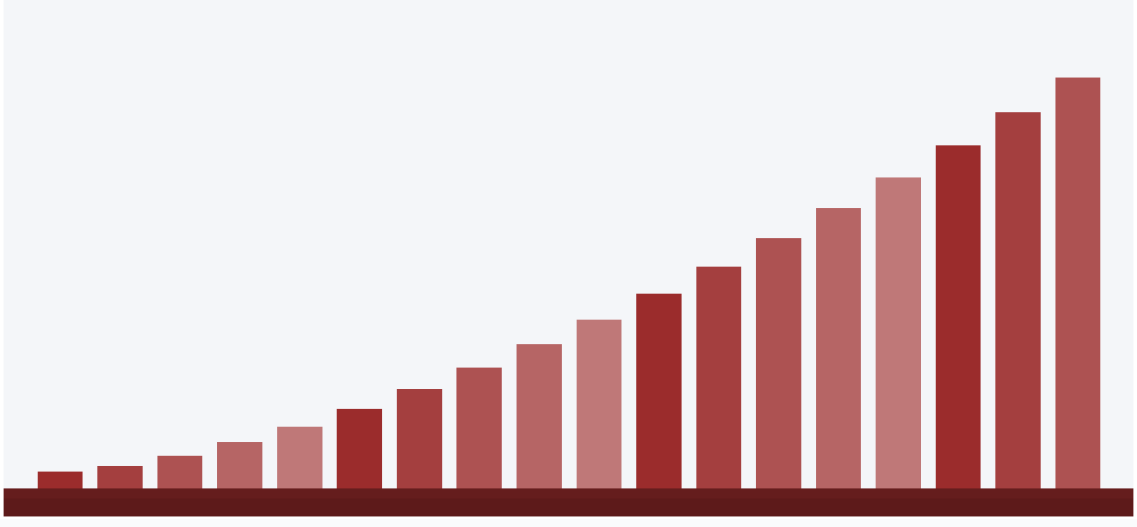
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Exposure, Modeling, and Behavioral Skills Training**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Exposure, Modeling, and Behavioral Skills Training to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 26

# Operant Conditioning and Contingency Management

Reinforcement, schedules, and the deliberate shaping of behaviour in clinical and educational settings.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the principles of operant conditioning and the four contingencies, and apply them to a real situation.
- Describe the schedule of reinforcement typology and specify an appropriate schedule for a given clinical objective.
- Explain the difference between reinforcement schedules and token economies, and between token economies and behaviour contracts.
- Describe the full procedure of contingency management, from the specification of the target behaviour to the review of the plan.
- Explain the design and the use of behaviour contracts, and the legal and ethical limits of contracts with minors.
- Apply differential reinforcement, shaping, and fading to a clinical problem.
- Describe the management of a behaviour that is maintained by escape, and explain why punishment so often fails.
- Plan a contingency management intervention in a school or clinic, including the training of the adults who will carry it out.
- Critically evaluate the ethical and cultural dimensions of contingency management with young people.

## Chapter Overview

Operant conditioning is the branch of the behavioural tradition that deals with behaviour that operates on the environment. The vocabulary it has produced — reinforcement, punishment, extinction, shaping, fading, the schedules — is the shared language of most of applied behaviour analysis, most of the behavioural interventions for parents, and a good deal of school practice.

Its central insight is that behaviour is selected by its consequences. This is a claim about the environment rather than about the person, and it immediately suggests an intervention: if a behaviour is selected by a particular consequence, then changing the consequence changes the behaviour. Nothing about the behaviour itself needs to be inspected, interpreted, or treated.

The second insight is that the world in which the young person lives is a set of contingencies, and that these contingencies are usually maintained by adults who are not aware of them. The task of the practitioner is to map them, and the mapping frequently produces surprises: the behaviour that has caused the most concern is often the behaviour that is currently being reinforced, and the cooperation that has been requested is often the cooperation that is being withheld.

The third insight is more technical and more consequential than it first appears: the *schedule* on which a consequence is delivered determines the behaviour it produces, and the schedule is the variable that a great many real-world interventions get wrong. A token economy that pays for a behaviour once a week produces a very different young person from one that pays for it every time it occurs.

This chapter presents the operant principles in their technical form, the schedule typology in the detail in which it is actually used, and the three principal clinical applications — differential reinforcement and

shaping programmes, contingency management in a setting, and behaviour contracts — with the adaptations that young people require and the ethical constraints that apply to each.

# 1. Historical Context

## 1.1 Thorndike's cats and the puzzle box

The operant tradition begins in Thorndike's puzzle box, and the shape of the experimental work determined the shape of the clinical practice that followed. A cat that stepped on a pedal and escaped received, in Thorndike's design, a fish; a cat that failed to step received nothing, or received an unpleasant consequence. The cat learned over time, and the learning curve recorded the improvement.

Three features of this design are the direct ancestors of contemporary practice.

**The consequence is immediate.** The fish followed the pedal press within seconds. Everything in the modern operant literature about immediacy, and about the superiority of a small immediate consequence over a large delayed one, follows from this.

**The behaviour is measured by its frequency.** Thorndike timed the escape, and the recording of a rate of responding under specified conditions is the foundational technique of the whole applied tradition.

**The learning is inferred from the environment, not from the animal.** Thorndike's law of effect made no reference to intention, satisfaction as a felt state, or purpose. The cat did not act in order to escape; it escaped because acting in that way had paid off.

## 1.2 Skinner and the operant

Skinner's contribution was to make the analysis of behaviour into a research programme and then into a discipline. Three elements of his system are foundational for clinical work.

**The operant is defined by its effect.** An operant is a behaviour that operates on the environment and produces a consequence. This definition makes the behaviour itself, and not the mental state, the object of analysis, and it makes the analysis of a behaviour identical to the analysis of the environment in which it occurs.

**The three-term contingency.** The behaviour is always specified in relation to a **discriminative stimulus** (the occasion on which the behaviour occurs) and a **consequence**. The full relation is $S \rightarrow R \rightarrow S$ : the antecedent signals that a particular response will be reinforced; the response occurs; the consequence follows. Clinical work with young people is, in the most useful sense, the specification and the alteration of three-term contingencies.

**The principle of the functional analysis of behaviour** — that behaviour is selected by its consequences and is therefore analysable in terms of the contingencies that produce and maintain it.

Skinner also introduced the systematic experimental apparatus, the operant chamber, in which behaviour is measured across a series of carefully manipulated conditions. The chamber is not a piece of equipment that a counsellor uses; it is the model of the clinical environment, in which the practitioner arranges conditions, measures behaviour, and manipulates consequences deliberately.

## 1.3 The transition to the clinic

The application of operant principles to human problems began in the 1940s and 1950s in three settings: the treatment of chronic mental illness in institutional settings, where the modification of Token Economy systems in the late 1950s was a landmark; the modification of classroom behaviour; and the parent-training literature of the 1960s, which developed the operant account of oppositional behaviour into the most practically influential behavioural treatment in the child mental health field.

The institutional work produced the first systematic contingency management programmes and, with them, the first ethical controversy, because the early Token Economy systems in psychiatric wards used the withdrawal of privileges as a primary tool and applied them inconsistently. The controversy led to the field's most durable contribution: the analysis of contingencies, which shows that a pattern of escalating behaviour is very often maintained by the escape contingency that the pattern itself creates.

The parent-training literature was the most consequential for the present book. The demonstration that oppositional and conduct behaviour in young children is frequently maintained by a pattern of adult escalation — the more the child escalates, the more the adult withdraws the demand, so that the escalation is rewarded — was one of the most practically useful findings in the entire behavioural literature, and it is the foundation of the parent training evidence described in Chapter 33.

### 1.4 Behaviour analysis in the schools

The application of operant principles in the classroom has a long history, and the three main approaches are worth distinguishing because they are still in use and they are often confused.

**Token economy** — a system in which desired behaviour is reinforced with tokens exchangeable for backup reinforcers, applied to a group or a class.

**Contingency management** — a negotiated restructuring of the reinforcement structure of a setting, in which the specific consequences for specific behaviours are defined, taught, and reviewed.

**Behaviour contracting** — a written agreement, between a young person and an adult or an organisation, specifying what each will do, with specified consequences, and with the young person's signature.

All three are forms of the same underlying operation, and all three appear in Chapter 28, which addresses their use in schools and clinics specifically.

## 2. Reinforcement in Clinical Practice

### 2.1 The specification of a reinforcer

A reinforcer is a stimulus that, when made contingent on a behaviour, increases the future frequency of that behaviour. The definition is behavioural, which means that the question "is this a reinforcer?" is answered empirically and not by appeal to what ought to be motivating. A young person may value an item that the practitioner considers trivial, and may be unmoved by an item that the practitioner considers valuable, and the practitioner's opinion is irrelevant to the question.

The practical consequence is that reinforcers must be established. Three methods are used together.

**Establishment by observation.** The young person is observed in a setting in which the behaviour in question is already frequent, and the consequences that follow it are recorded. A young person who talks continuously in one class and is rewarded with attention there has a reinforcer available.

**Establishment by interview.** The young person is asked directly what they want, what they like, what they will work for, what they would spend their money on, what they would do for nothing. The answers are taken seriously even when they seem trivial.

**Establishment by testing.** A small amount of the candidate reinforcer is delivered, and the effect on the behaviour is observed. This is the only method that settles the question, and it is under-used.

### 2.2 The reinforcer survey

The **reinforcer survey** is a structured interview in which the young person, the parents, and the teachers are each asked the same set of questions about what the young person likes, wants, finds interesting, spends time on, and will work for. The three sets of answers are compared.

The comparison frequently produces the most useful single finding in a behavioural assessment. The young person wants X; the parents are willing to give Y; the teacher can actually deliver Z. Where these three sets do not intersect, the plan will not work, and the discovery is usually made only by asking all three.

### 2.3 Natural reinforcers and the motivational operations of the setting

The most durable plans use consequences that are already part of the young person's life. A child who enjoys drawing will work for drawing time far more reliably than for a token exchangeable for sweets, because the drawing does not have to be procured, scheduled, and administered, and because the value of it does not decline with repetition in the way that the value of an administered treat does.

This is the practical form of the account of **motivating operations** given in Chapter 23. The relevant question is not "what does this young person like?" but "what is scarce in this young person's current environment, and what would be abundant if it were provided?" Scarcity is what makes a stimulus reinforcing, and the practitioner's best move is often to provide something that is plentiful in the young person's world outside the clinic and scarce in the clinic.

### 2.4 Reinforcement and the young person's dignity

A systematic and sustained programme of reinforcement has an obvious dignity cost, and the practitioner should anticipate it. A young person who is rewarded for talking, and paid for it in tokens, may learn to talk for the tokens and may find the relationship with the practitioner humiliating.

Four practices reduce the cost without reducing the effectiveness.

**Reinforce the behaviour, not the person.** The young person earns the consequence for the behaviour, and the practitioner never indicates that the value of the young person is conditional.

**Use a range of reinforcers**, so that the interaction is not a transaction.

**Move towards natural consequences** as rapidly as possible, because natural consequences carry no dignity cost at all and generalise better.

**Fade the artificial system deliberately**, as a planned phase of the treatment, rather than abandoning it, which is experienced as a loss.

## 3. Schedules of Reinforcement

## 3.1 The four schedules

The schedule on which a reinforcer is delivered determines the behaviour it produces. Ferster and Skinner's typology is the standard description, and it remains the most useful single tool in the behavioural repertoire for matching a contingency to an objective.

**The fixed-ratio (FR) schedule** delivers the reinforcer after a fixed number of responses. FR 5 means the fifth response is reinforced, and then the count restarts. The characteristic pattern is a **post-reinforcement pause** followed by a high, steady rate: the pause after the reinforcement, then rapid responding until the next reinforcer. FR schedules produce high rates of response with brief pauses.

**The variable-ratio (VR) schedule** delivers the reinforcer after a variable number of responses, unpredictable to the organism. The characteristic pattern is a high, steady rate with no pauses, and this is the schedule that produces the most persistent behaviour. Gambling, and the use of slot machines, are the familiar examples, and the reason that a gambling schedule is so resistant to extinction is that the organism cannot distinguish a long run of losses from a schedule that has merely been quiet.

**The fixed-interval (FI) schedule** delivers the reinforcer for the first response after a fixed time period has elapsed. The characteristic pattern is a **scallop**: a pause after the reinforcement, then a gradually accelerating rate of response as the time approaches. FI schedules produce moderate rates with a distinctive temporal pattern, and they are the schedule that generates the "waiting until the last moment" behaviour that teachers and employers find so familiar.

**The variable-interval (VI) schedule** delivers the reinforcer for the first response after a variable time period. The characteristic pattern is a moderate, steady rate with no pauses, and this is the schedule that produces the most persistent and most stable behaviour of the four.

## 3.2 The schedule and the clinical objective

The typology is directly actionable, and the following table is the working summary that a student should carry into practice.

| Schedule | Behaviour produced | Clinical application |
|---|---|---|
| Fixed ratio | High rate, brief pause after reinforcement | Token systems, work-based reinforcement, discrete-trial programmes |
| Variable ratio | High, steady rate, highly persistent | Attendance in a group, completing a session, sustained effort tasks |
| Variable interval | Moderate, steady rate, most stable | Ongoing attention, monitoring, being available, homework checking |
| Fixed interval | Moderate rate, scalloped, "last minute" | Periodic check-ins, scheduled reviews, homework collection |

Two clinical consequences follow immediately. The first is that a *daily* reward for a behaviour that occurs many times per day is a fixed-ratio schedule and produces a pause after each reward, and a practitioner who has set a token economy with a small daily bonus will see the young person do the behaviour in a burst at the end of the day and not at all during it. The second is that a young person who is checked "every week" will do the work at the end of the week, and the fix is not more motivation but a shorter interval.

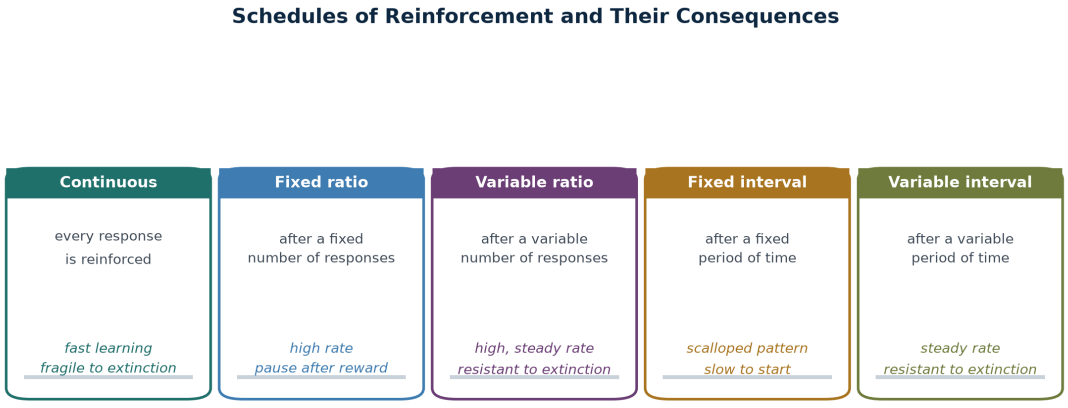



**Figure 26.15** The four schedules of reinforcement compared, showing the characteristic response patterns for fixed ratio, variable ratio, fixed interval, and variable interval schedules over time.

## 3.3 Reinforcement for acquisition and for maintenance

The schedule used during acquisition is not the schedule used during maintenance, and confusing the two is a common design error.

**Acquisition** is best served by a **continuous reinforcement** schedule: every instance of the behaviour is reinforced. The risk is that the behaviour becomes dependent on the reinforcement, and that the young person has not learned to perform it in the absence of the contingency.

**Maintenance** requires moving to a **thinning schedule**, in which the reinforcer is delivered less and less frequently, while the behaviour continues. The thinning schedule is the mechanism by which the behaviour becomes self-sustaining, and the rule of thumb is that the behaviour must be well established and consistent before thinning begins.

**The thinning rule.** The schedule should be thinned only after the behaviour has been established at a high and consistent rate, and the thinning should be gradual rather than abrupt. A behaviour that is thinned too early extinguishes, and the practitioner will misread the extinction as evidence that the behaviour is not under the young person's control.

**The transition to natural consequences.** The final stage is the transfer of the behaviour to the natural contingencies of the young's life, and the goal of the whole procedure is to reach it.

## 3.4 The extinction of over-learned behaviour

Extinction is not always the goal, and the practitioner should distinguish three situations.

**Extinction is appropriate** when the behaviour should stop and no alternative is required — the punching, the shouting, the out-of-seat behaviour that has no function other than its own continuation.

**Extinction is inappropriate** when the behaviour is positively reinforced. A young person who is being praised for elaborating on their day will not stop elaborating, and the attempt to extinguish the talking will produce a marked increase in the talking, which is the **extinction burst** and which the young person and the practitioner will both find alarming.

**Extinction is dangerous** when the behaviour is maintained by negative reinforcement. This is the case that most requires a functional analysis, and it is the subject of Section 5.

### 3.5 Differential reinforcement

**Differential reinforcement** is the reinforcement of one behaviour in a way that distinguishes it from another, and it is the technique of choice wherever a behaviour must reduce without the relationship deteriorating.

Three forms are used.

**Differential reinforcement of other behaviour (DRO)** — reinforcing any behaviour other than the target. It is the least specific form and is used when the target behaviour is dangerous and must be suppressed immediately, as in the case of self-injury.

**Differential reinforcement of an incompatible behaviour (DRI)** — reinforcing a behaviour that cannot be performed at the same time as the target. A young person who is learning to keep their hands occupied is reinforced for keeping their hands occupied, which is incompatible with hitting.

**Differential reinforcement of the target behaviour (DRT or DRA)** — reinforcing the target behaviour in the presence of the target, and reinforcing an alternative behaviour in its absence. This is the form that produces the most durable change, and it is the form most often required when a young person is working on a skill that is difficult in the situations in which it is most needed.

Differential reinforcement has a further advantage that matters in a clinical relationship: it is a way of reinforcing a young person *without* reinforcing the problem, and it therefore permits the practitioner to maintain a positive relationship with the young person at the same time as working on the problem behaviour.

## 4. Shaping and Fading in Clinical Use

### 4.1 Shaping revisited

The procedure was described in Chapter 23 and is repeated here in its clinical form, because the clinical details are what make it work.

**Specify the target behaviour precisely.** The written specification should describe what the young person will do, how the practitioner will know it has occurred, and what the target rate or duration is.

**Identify the starting approximation.** This should be a behaviour the young person already does, at least occasionally, and reliably enough to be reinforced.

**Order the approximations.** The ordering should reflect the difficulty, not the logical sequence. A common error is to order the approximations as the steps in the physical task, which ignores the fact that an earlier step may be the difficult one.

**Specify the reinforcer for each approximation**, and make it immediate.

**Define the advance criterion** — the number of consecutive correct performances at one approximation that justifies moving up. Without an advance criterion, the practitioner moves up when they feel ready, and the behaviour degrades.

**Define the step-back rule** — the conditions under which the practitioner returns to the previous approximation. This is the safeguard against the "too good to last" pattern in which a practitioner pushes a young person forward, the behaviour deteriorates, and the whole plan is abandoned.

**Define the maintenance criteria** — when the plan moves from shaping to thinning.

### 4.2 Prompting and fading

A prompt is any stimulus that assists the initiation or performance of a behaviour, and the fade plan is not optional. The hierarchy runs from the least intrusive to the most: a natural cue, a gesture, a verbal direction, a visual prompt, a modelling demonstration, a physical or hand-over-hand prompt, and finally a physical prompt.

**The fade plan must be written down before the prompt is introduced.** The fade is slower than the practitioner expects, and a plan that is not written down will be abandoned on the day.

**Fading is usually slower for a young person with a learning difficulty or an intellectual disability**, and the fade criteria should be adjusted before the plan begins rather than during it, because a young person whose behaviour deteriorates during a failed fade experiences it as failure and the relationship is damaged.

**Fading is faster in a natural setting than in a clinic**, and the transfer of the prompt from the clinic to the school is a specific problem, because the school staff will not have been trained in the prompt hierarchy.

### 4.3 Prompt dependence and its prevention

A behaviour that is maintained by a physical or a verbal prompt is not the behaviour the practitioner wanted, and the signs of prompt dependence are recognisable. The young person performs the behaviour only when prompted; the performance does not increase across sessions; and the behaviour disappears when the prompt is faded.

The prevention is procedural: fade systematically, confirm that the behaviour persists after the fade, and treat a failed fade as information about the approximation rather than as a failure of the young person.

## 5. Escape-Maintained Behaviour

Escape-maintained behaviour is the most important single category in applied practice, and the management of it is the point at which the operant analysis earns its keep.

### 5.1 The pattern

A young person who is asked to do something and who escalates — arguing, shouting, breaking something, running — is performing a behaviour that reliably terminates the demand. The escalation is not a failure of the behaviour; it is the behaviour working. The more severe the escalation, the faster the demand ends, and the more reliably the behaviour is reinforced.

The circularity is exact: the adult's response to the escalation is the maintaining contingency, and the adult's response is usually made in the belief that the escalation is the problem.

### 5.2 The clinical procedure

The management of escape-maintained behaviour proceeds in a defined order, and the order matters.

**Step 1: Establish that the behaviour is escape-maintained.** This requires data, and the data come from the ABC record, the incident log, and the interview. The practitioner looks for the pattern: escalation followed by withdrawal of the demand, and consistency of that pattern.

**Step 2: Identify the demand.** What is the specific demand, and what is the young person's response to it? A young person who cannot tolerate a demand they do not understand will escalate, and the escalation may be a request for help.

**Step 3: Plan the response.** The adult's response to the escalation is the treatment, and the plan must specify it in advance, in writing, and it must specify it for the whole team, because the plan fails if one adult in the corridor responds differently.

**Step 4: Teach an alternative.** A young person cannot be expected to comply with a demand that has no tolerable alternative. The alternative is usually a request for help, a break in a specified form, or a graded reduction in the demand, and it must be made available at the same time as the demand.

**Step 5: Fade.** The demand is increased gradually, and the alternative is faded as the compliance is established.

### 5.3 The differential reinforcement of compliance

**Differential reinforcement of compliance** — reinforcing compliance with a demand and ignoring escalation — is the core technique. It must be implemented with care, because the "ignoring" must be genuine: the practitioner must not look away, not speak, and not give any signal, because a signal of attention is a reinforcer.

The technique has a predictable course and the practitioner should prepare the young person for it. The behaviour increases in the first few sessions, because the young person is testing and the testing produces a burst. Then it declines, sometimes sharply, and the decline is a sign that the extinction is taking hold rather than a sign that the plan has failed.

### 5.4 Why punishment fails here

Punishment of escape-maintained behaviour fails systematically, and the reason is worth stating because it is the most practically useful thing in this section.

Punishing the escalation does not teach compliance; it teaches that the escalation has a cost, and the young person learns to escalate in a way that is less detectable or less costly. Meanwhile, the escape remains available and the avoidance behaviour, which is the behaviour that is actually maintaining the problem, is untouched. And because the practitioner has escalated the response, the young person has learned that adults in this situation become larger, which may increase the aggression in other settings.

The correct target is the avoidance, and the correct method is to make the demand tolerable and to reinforce the compliance, not to make the escalation expensive.

## 6. Further Reading for This Chapter


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- Milner, P. M. (1953). *The behaviour of the rat*. — The laboratory reference.
- Leifer, M. L. (2001). *Teaching a language of behavior*. — An accessible clinical orientation.
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# 7. Contingency Management in Clinical Settings

## 7.1 The definition

**Contingency management** is the deliberate and negotiated restructuring of the reinforcement structure of a setting, in which the specific consequences for specific behaviours are defined in advance, taught to everyone involved, and reviewed at stated intervals. It is broader than a token economy, which is one technique within it, and broader than a behaviour contract, which is another.

The defining features of contingency management are that it is **specified** rather than improvised, that it is **negotiated** with the people whose behaviour it governs, that it is **transparent** — everyone knows the rules — and that it is **reviewed**, with a stated end point, rather than being left to run indefinitely.

## 7.2 The procedure

**Step 1: Specify the target behaviours.** In a clinical setting, a small number of clearly defined behaviours, chosen because their change matters. A common clinical specification is: attending sessions; completing a specified task; not using substances during the session; contacting the practitioner between sessions.

**Step 2: Specify the consequences.** For each target behaviour, the consequence that follows it and the consequence that follows its absence. Both directions must be specified, because a contingency that only defines rewards is not a contingency.

**Step 3: Identify the reinforcers.** By the survey method of Section 2, and by testing the candidates.

**Step 4: Specify the schedule.** The interval and ratio, and the thinning plan.

**Step 5: Specify the rules for the exceptions.** A contingency that does not specify what happens when the rules are broken cannot be administered, and the specification should cover illness, absence, crisis, and the days when the young person is not capable of complying.

**Step 6: Train the staff.** Every person who will deliver a consequence must know the rules, and the training should be a rehearsal rather than a briefing. This step is the one most often omitted and the one most responsible for the failure of contingency management programmes.

**Step 7: Review at stated intervals.** A contingency that is not reviewed becomes an unexamined routine, and unexamined routines drift.

**Step 8: Fade and terminate.** Every contingency management programme should have a planned end. The goal of a token economy is not a young person who needs tokens; it is a young person who no longer needs the system, and a system with no end point is a system that has become the plan.

### 7.3 Contingency management with a young person in difficulty

The context of a young person's life makes contingency management unusually delicate, for three reasons.

**The young person's environment is shared.** The school, the home, and the clinic are three environments with three sets of contingencies, and if the three do not align, the young person will select the environment with the most generous contingencies, which is usually neither the one that is best for them nor the one they live in.

**The stakes of the consequences are uneven.** A young person in a foster placement or in a residential setting may be subject to a set of consequences that were never negotiated, some of which are severe and some of which are applied inconsistently, and the practitioner who introduces a negotiated system into that environment may find that the negotiated system is the least powerful one in the young person's life.

**The consequences are often experienced as unjust.** A young person who is used to a system in which the rules change according to the mood of the adult will experience a transparent system as a novelty, and the novelty is usually welcome, and the welcome is itself useful data.

### 7.4 The negotiated alternative

The **negotiated alternative** is a technique developed in the family literature and adopted more widely than it deserves. In it, the young person is asked to propose what they would be willing to do in exchange for what they want, and the practitioner negotiates from the young person's proposal rather than from a preset offer.

The technique is powerful because it produces a commitment, and it is limited because the young person's proposal will usually be worse for the practitioner than a preset offer, and the pressure to accept it is difficult to resist. The balanced practice is to take the young person's proposal seriously, to state the constraints honestly, and to make the final agreement one that the practitioner could actually deliver. A young person who has negotiated a reward the clinic cannot provide has been set up to fail, and the failure will be attributed to the young person.

## 8. Behaviour Contracts

## 8.1 The structure

A **behaviour contract** is a written agreement specifying what each party will do, and the consequences of their doing or not doing it. The standard elements are: the parties; the specific behaviours promised by each party; the duration of the agreement; the specified consequences; the review date; and the signatures.

The feature that distinguishes a contract from a token economy is **reciprocity**: the adult also makes promises, and the consequences apply to the adult as well. A contract in which the young person promises to attend three sessions and the practitioner promises to be there at the agreed time, with a stated consequence if the practitioner is not, is a different instrument from one in which only the young person is bound.

Reciprocity also serves a clinical purpose. It converts the young person from the position of a subject of the system to the position of a participant in it, and the young person who has a say in the design of a contract is markedly more likely to be honest about it in the review.

## 8.2 Contracts with minors

A contract with a young person is an agreement and not a legal instrument, and this must be clear. A contract with a minor cannot be enforced in the way an employment contract can, and a practitioner who presents it as binding is misrepresenting its status.

Three limits should be observed.

**The consequences must be within the adult's power to deliver.** A consequence that depends on another authority — a school's attendance policy, a benefit agency, a court — is not a contractual consequence and should not be written as one.

**The contract must not contract for the removal of a young person's rights.** A contract that trades a young person's right to see a friend, or their right to privacy, or their right to complain about a staff member, for behaviour is an abuse of power regardless of how it is framed, and no behavioural formulation justifies it.

**The contract must be voluntary.** A young person who is required to sign a behaviour contract as a condition of continued attendance in a school or a service has not entered into an agreement.

## 8.3 The uses of a contract

Contracts are well suited to a young person who is older, who is able to understand the terms, and whose behaviour is a matter of a stable disagreement rather than of a clinical symptom. They are particularly useful in three situations.

**Ambivalence about treatment.** A young person who is ambivalent about therapy can be helped to commit to a defined and limited number of sessions through a contract, and the contract makes the commitment concrete and reviewable.

**Ambiguity about what is expected.** A young person who does not know what is expected of them cannot be held to it, and the contract specifies it.

**Conflict between a young person and a school.** A contract between a young person and a school, with the school also bound, can change the terms of an argument that has become entrenched.

Contracts are poorly suited to young children, to young people in acute crisis, and to situations in which the central difficulty is the young person's safety.

### 8.4 The review

Every contract must have a review, and the review is the part that most often does not happen. The review is a shared examination of what happened, and it is where the agreement is amended, extended, or ended. A contract reviewed only when it breaks down is not a contract; it is a set of threats.

## 9. Case Illustration: The Young Person Who Would Not Eat

**Presentation.** Ahmed is eleven and has been referred by his school because he has been refusing to eat in the dining hall, and he now eats a small amount at home before school. He is very thin, and his mother is frightened. The school has tried a variety of approaches, including a "healthy lunch club" for children who are not eating, and a reward chart for "eating a whole sandwich", and both have failed.

**The assessment.** The behavioural assessment establishes three things.

*The behaviour.* Ahmed does not eat in the dining hall. At home he eats, and he eats a restricted range. The mother reports that he eats pasta, rice, plain chicken, and nothing else, and that this has been the case for two years, and that it started after a year in which he was "sick" and "couldn't keep food down".

*The antecedents.* The observation in the dining hall is unremarkable in a way that is itself informative. Ahmed sits with a group of boys, he does not talk much, nobody is attending to him, and the food in front of him is disliked. There is no peer pressure, no coercion, and no obvious social consequence. He simply does not eat.

*The consequences.* When he does not eat, nothing happens. He leaves the dining hall, goes to the corridor, and is alone for twenty minutes, and he is not spoken to.

**The hypothesis.** The behaviour is maintained by the **avoidance of the dining hall**, and the avoidance is maintained by the relief of leaving, and — this is the crucial finding — the young person has no experience of what eating there would be like, because he has not done it for two years. The condition that the avoidance prevents is not "eating" but "eating in that room, with those people, with that food."

There is a second and more serious question, which the behavioural assessment raises and must not conceal. The restriction began after a period of illness in which he could not keep food down, and the behavioural account cannot say what that illness was, whether there was vomiting, whether there was fear of vomiting, or whether the restriction is now maintained by something that is not a contingency at all. The practitioner must not proceed with the behavioural plan until the medical history has been established, and the referral to the paediatric service must be made first, because a restrictive eating pattern in an eleven-year-old requires medical assessment before any behavioural intervention.

**The plan.** The plan has two stages, and the order matters.

*Stage one: the medical and the assessment of the eating disorder.* The paediatric assessment, the eating disorder screening, and the family work that supports it. The behavioural work is deferred.

*Stage two: the graded return to eating in the dining hall.* This is an exposure and a shaping programme together, and it uses the principles of Chapter 25.

The hierarchy is built with Ahmed, and it is built in the small dimensions rather than the large one. The steps are: sit in the dining hall for two minutes with a plate of food and leave; sit for five minutes; sit with one friend; have a drink; have a sandwich and leave; have a sandwich and stay for the whole lunch period.

The reinforcer for each step is time in the corridor with his friend afterwards, which the school can deliver and which the reinforcer survey confirms is what Ahmed values.

The shaping is at the level of a single sandwich, and the advance criterion for each step is three consecutive successes.

The **fading** is built in from the start. The school agrees that the "healthy lunch club" arrangement ends, and that Ahmed returns to the dining hall rather than being assigned to a separate space, because a separate space removes the very cue that is being treated. The school also agrees that the adults do not comment on his eating, do not encourage, do not praise, and do not remark when he does not eat. The comments that were intended as encouragement are, in the operant analysis, attention that is contingent on the behaviour, and the praise for "eating a whole sandwich" is positive reinforcement of a behaviour that is reinforced only in that setting and nowhere else, which is why the reward chart produced no durable change.

**The family work.** Ahmed's mother is asked to make two changes. She stops cooking around his restriction and begins to serve the same food for the family, without comment, without negotiation, and without a separate "safe" meal for him. And she stops the "try just a little bit" routine, which is the most common and least effective of the responses to a restricted eater, and which has the effect of making every meal a negotiation in which the food becomes the arena.

The mother's anxiety about this plan is substantial, and it should be, and the practitioner should treat it as the central clinical task rather than as a compliance issue. A mother who is frightened of her son's health is being asked to stop the thing that is currently reducing her anxiety, and the anxiety will find another outlet. The plan therefore includes a regular appointment with the mother for as long as it takes.

**The outcome.** Ahmed progresses through the hierarchy over approximately eleven weeks, with two relapses, one following an illness and one following a conflict at home. The restriction at home narrows over the same period, which is expected and which is a useful sign. His weight improves slowly. The paediatric service remains involved, and the multidisciplinary plan is reviewed monthly.

**The interpretation.** The case illustrates the discipline's central clinical discipline: the question was not how to make Ahmed eat but what his not eating was accomplishing, and the answer — the relief of leaving a room he was not ready to be in, maintained by a contingency nobody had designed — changed the intervention entirely. It also illustrates the discipline's central clinical danger: the behavioural analysis was elegant, and it was the wrong analysis, because the behaviour had a medical cause that the contingency analysis could not see. The practitioner who had proceeded directly to the graded exposure would have treated a young person's illness as a habit.

# 10. Evidence and Research Support

## 10.1 The status of the evidence

The evidence base for operant interventions is large, and it is strongest where the intervention is specified, the behaviour is operationally defined, and the outcome is measured. The most important evidence findings are as follows.

**Parent training based on operant principles has a strong evidence base** for oppositional behaviour, conduct disorder, and disruptive behaviour in the school setting, and this is among the most replicated findings in the child behavioural literature. The Cochrane reviews of parent training for conduct disorder

and oppositional behaviour report substantial and durable effects, and the findings are among the most robust in the entire child mental health evidence base.

**Token economies have strong evidence for institutional and educational settings**, particularly for the reduction of disruptive behaviour in classes of students with behavioural difficulties, and the effect sizes in the meta-analyses are large.

**Differential reinforcement has strong support** for the reduction of specific behaviours and for the teaching of alternatives, and it is a component of nearly every successful behavioural programme.

**Schedules of reinforcement have strong experimental support**, and the translation of the schedule findings into clinical practice is well established in the applied literature.

**Contingency management for substance use has some of the strongest effects in the entire psychological literature**, with effect sizes for the treatment of stimulant and opioid use that are larger than those for any other treatment for these conditions, and Chapter 47 develops the ethical and practical questions that the strength of these effects raises.

## 10.2 The evidence with young people specifically

Three findings are specifically relevant to this book.

**Parent-delivered contingency procedures outperform practitioner-delivered procedures** for the younger age range, in which the contingencies that matter are in the home and the parents are the only people who can change them.

**The fidelity of the implementation determines the outcome**, and the studies that measure fidelity and find it low report null results that reflect the implementation rather than the intervention.

**Contextual factors moderate the effect.** Parent training works better when the child has a comorbid condition, which is a counterintuitive and well-replicated finding that is best explained by the observation that the presence of more severe behaviour increases the parents' motivation and the salience of the training.

## 10.3 Limitations of the evidence

**The literature is dominated by young children and oppositional behaviour.** The evidence for contingency management with adolescents, particularly in outpatient settings, is thinner than the evidence with parents of young children, and the field's adolescent applications are more often extrapolated than demonstrated.

**The outcomes are almost always behavioural**, and the measures of the young person's own experience of the intervention, of the family's experience, and of the durability of the change in naturalistic settings are much less systematically reported. This is the gap that the 1993 revisions of the applied research criteria were directed at, and it is still not fully closed.

**Many of the studies are conducted in settings with distinctive features** — residential schools, specialist classrooms, hospital wards — and the generalisation to a general school or a general outpatient clinic is a real question.

**The allegiance effects are substantial.** Where the researcher is a member of the behavioural tradition, the effects are larger, and this pattern is one of the most consistent findings in the entire evidence base, described in Chapter 7.

# 11. Strengths and Limitations of Contingency Management

## 11.1 Strengths

**It is specified, teachable, and reproducible.** A contingency management programme can be written down, handed to a team, and implemented with fidelity, and this is not true of most psychotherapies.

**It addresses the environment rather than the person.** The intervention is directed at the contingencies, which are in the adult's gift, and this is a considerable advantage where the young person's internal states are not.

**It produces rapid, measurable change.** Contingency management is often the most rapidly effective intervention available for a specific behaviour, and rapid change is important for a young person who is demoralised by a long history of unsuccessful attempts.

**It preserves the relationship.** Differential reinforcement permits the practitioner to reinforce the young person while working on the problem, and this is a substantial advantage over approaches that require the practitioner to withhold attention.

**It is portable across settings**, and it can be implemented by parents, teachers, and ward staff, with training.

## 11.2 Limitations

**It requires resources and consistency.** The programme requires trained staff, a stable environment, and the sustained attention of a practitioner. In a service with high turnover and low continuity, the failure is a failure of implementation and will be misread as a failure of the approach.

**It can be experienced as coercive.** A young person who is being managed by a written system of rewards and consequences may experience it as the loss of personal agency, and in some cases the experience will be accurate. The ethical section addresses this directly.

**It addresses the behaviour and not the context that produced it.** A contingency programme that reduces a young person's offending behaviour while leaving the conditions that generated it untouched has solved a symptom, and the practitioner should be alert to the possibility of a displaced problem.

**It can create dependence on the system**, and the fade plan is not optional.

**It says nothing about the young person's inner life**, and a young person whose behaviour is maintained by shame, by self-hatred, or by a conviction of their own unworthiness will not be helped by a system that addresses the contingencies of the behaviour.

# 12. Cultural and Ethical Considerations

## 12.1 The reward and the market

The contingency management framework is often criticised for importing a market model into relationships, and the criticism has a real core. A young person who learns that affection, attention, and privilege are commodities that must be earned by performance has learned something that is not only false but corrosive, and the literature on the effects of reward-based parenting is the empirical counterpart of the moral objection.

Four principles follow for the practitioner.

**Never contract for what should not be traded.** Basic attention, genuine warmth, and access to help should not be contingent on performance. The practical form of this is that a practitioner must not withdraw attention or warmth as a consequence, and must not ration approval.

**Reinforce the behaviour, never compliance with the practitioner.** The distinction is subtle and important: reinforcing a young person's honest report, or their effort, or their self-regulation, is different from rewarding them for agreeing with the practitioner.

**Use natural consequences wherever possible.** A natural consequence carries no dignity cost and generalises better, and the progression from artificial to natural is a progression towards autonomy.

**Be aware that the system changes the relationship.** A young person who is being managed by a token system will behave differently in the sessions, and the practitioner should be honest with themselves about the fact that some of the improvement observed in session is contingent on the session.

## 12.2 Cultural considerations

**Reinforcers are cultural.** The reinforcer survey must be conducted in the young person's own frame of reference, and the practitioner must expect that the young person's values may differ from the family's, from the school's, and from the practitioner's. In a context where material rewards are scarce, a token may be a very powerful reinforcer; in a context where material rewards are abundant, it may be a very weak one, and the practitioner who has assumed otherwise will build a plan on a contingency that does not exist.

**The family's own systems of reward and sanction are already operating.** A family in which compliance is rewarded with togetherness and independence is rewarded with freedom operates a contingency system, and a clinic-based programme that runs counter to it will be defeated by it. The most useful clinical question is not "what reinforcer will work?" but "what does this family already reward, and how can the plan be built on that?"

**The honour and the stigma of the systems.** In many communities, a token economy or a written contract introduces a public record of the young person's behaviour that is experienced as a humiliation. Where the young person's peers can see the system, the design must protect the young person's dignity, and in some settings the system should be invisible to peers.

**The authority to set contingencies.** In some settings, the assumption that a professional has the standing to determine what a young person will be rewarded for is itself a cultural assumption, and the resistance it generates is rational and should be understood on its own terms.

## 12.3 Ethical requirements

**Informed assent and the right to withdraw.** A young person must be able to decline to participate in a contingency programme, and the consequences of declining must be stated in advance. A young person who is in a residential or a school setting where the programme determines some of their access must be able to decline without penalty.

**Proportionality.** The magnitude of the consequences must be proportionate to the situation. A young person in a residential setting should not have a disproportionately large stake in a system over behaviour that is trivial.

**The prohibition on contingent aversive procedures.** The contemporary practice does not use contingent aversive stimuli.

**The prohibition on the deprivation of a young person's rights.** No system may trade access to education, to healthcare, to food, to rest, to privacy, to advocacy, or to complaint for behaviour.

**Protection against the failure of the system.** A contingency programme that a young person cannot meet has failed, and the response to the failure must be an adjustment of the programme rather than an increase in the consequences. The principle should be stated to the young person in advance: if the plan is not working, the plan changes.

**Documentation and review.** The programme, the rules, the review points, and the outcomes should be documented, and the young person should be able to see the record.

## 13. Key Terms

### Key Terms at a Glance

- **Operant** — Behaviour that operates on the environment and is selected by its consequences; distinguished from the respondent, which is elicited by a stimulus.
- **Three-term contingency** — The relation in which a discriminative stimulus signals that a response will be reinforced, the response occurs, and a consequence follows.
- **Discriminative stimulus (S )** — The antecedent stimulus that signals that a particular response will be reinforced and that another will not.
- **Consequence (S )** — The event that follows a response and that determines the future probability of that response.
- **Reinforcer** — A stimulus that, when made contingent on a behaviour, increases the future frequency of that behaviour.
- **Reinforcement value** — The degree to which a stimulus is preferred, which determines the amount of behaviour that can be acquired and maintained by its delivery.
- **Motivating operation** — An event that establishes or abolishes the reinforcing effectiveness of a stimulus.
- **Natural reinforcer** — A consequence that is already part of the young person's environment, requiring no administration and generalising well.
- **Reinforcer survey** — A structured interview with the young person, the parents, and the teachers, each asked the same questions about what the young person values, for the purpose of identifying a usable reinforcer.
- **Fixed-ratio (FR) schedule** — Reinforcement delivered after a fixed number of responses; produces a high rate with a post-reinforcement pause.
- **Variable-ratio (VR) schedule** — Reinforcement delivered after a variable number of responses; produces a high, steady, and highly persistent rate with no pauses.
- **Fixed-interval (FI) schedule** — Reinforcement delivered for the first response after a fixed time period; produces a scalloped response pattern.
- **Variable-interval (VI) schedule** — Reinforcement delivered for the first response after a variable time period; produces a moderate, steady, and very stable rate.
- **Scallop** — The characteristic acceleration of responding across a fixed interval as the reinforcement time approaches.
- **Post-reinforcement pause** — The brief cessation of responding that follows a fixed-ratio reinforcement.

## Key Terms at a Glance (continued)

- **Continuous reinforcement** — Reinforcement of every instance of a behaviour; appropriate to acquisition and unmaintainable as a permanent arrangement.
- **Thinning schedule** — The gradual reduction in the frequency of reinforcement, used to transfer control from the schedule to the natural environment.
- **Shaping** — The systematic reinforcement of successive approximations to a target behaviour.
- **Advance criterion** — The specified number of consecutive correct performances required before moving to the next approximation.
- **Step-back rule** — The specified conditions under which the practitioner returns to a previously successful approximation.
- **Prompt** — A stimulus that assists the initiation or performance of a behaviour.
- **Prompt hierarchy** — The ordering of prompts from least to most intrusive: natural cue, gesture, verbal direction, visual prompt, modelling, hand-over-hand, physical.
- **Fading** — The systematic withdrawal of a prompt across successive trials, with reinforcement transferred to the unprompted behaviour.
- **Prompt dependence** — A state in which the behaviour occurs only when the prompt is delivered and does not persist when the prompt is faded.
- **Differential reinforcement of other behaviour (DRO)** — Reinforcement of any behaviour other than the target behaviour.
- **Differential reinforcement of an incompatible behaviour (DRI)** — Reinforcement of a behaviour that cannot be performed at the same time as the target behaviour.
- **Differential reinforcement of other behaviour and the target (DRT/DRA)** — Reinforcement of the target behaviour in the presence of the target stimulus and of an alternative in its absence.
- **Differential reinforcement of compliance (DRC)** — Reinforcement of compliance with a demand, combined with the extinction of escalation.
- **Extinction** — The discontinuation of reinforcement, producing a decline in the behaviour.
- **Extinction burst** — A temporary increase in the frequency or intensity of behaviour at the beginning of extinction.
- **Escape-maintained behaviour** — Behaviour that is maintained by the removal or postponement of a demand; the escalation is the behaviour working, not failing.
- **Contingency management** — The deliberate and negotiated restructuring of the reinforcement structure of a setting, with specified consequences, trained staff, and stated review points.

**Key Terms at a Glance (continued)**

- **Token economy** — A system in which desired behaviour is reinforced with tokens exchangeable for backup reinforcers.
- **Backup reinforcer** — The item or privilege for which tokens are exchanged.
- **Behaviour contract** — A written agreement specifying what each party will do and the consequences, with a review date and signatures.
- **Reciprocity in a contract** — The principle that the adult also makes promises and is also bound by the stated consequences.
- **Negotiated alternative** — A technique in which the young person proposes what they will do in exchange for what they want, and the agreement is negotiated from that proposal.
- **Contingency fading** — The planned and staged withdrawal of a contingency as the behaviour becomes self-sustaining.
- **Antecedent behaviour consequence (ABC) data** — The observational record of the sequence surrounding a behaviour, used to generate and refine functional hypotheses.
- **Dignity cost** — The interpersonal cost to the young person of being managed by a reward and consequence system.
- **Contingency management for substance use** — The application of reinforcement principles to the treatment of substance use disorders, with unusually large measured effects.
- **Fidelity of implementation** — The extent to which an intervention is delivered as designed, measured and reported alongside the outcome.

## 14. Chapter Summary

Operant conditioning is the branch of the behavioural tradition concerned with behaviour that operates on the environment, and its central claim is that behaviour is selected by its consequences. The claim is simultaneously the theory, the method, and the clinical proposition: if a behaviour is selected by a particular consequence, then changing the consequence changes the behaviour, and nothing about the behaviour itself needs to be interpreted in order to intervene.

The chapter traced the lineage from Thorndike's puzzle box, through Skinner's three-term contingency and functional analysis, into the ward-based token economies of the 1950s, the parent training literature, and the classroom applications. The specification of a reinforcer was treated as a clinical task requiring three methods — observation, interview, and testing — and the reinforcer survey was presented as the most economical way of finding a usable reinforcer, with the important observation that the young person's, the parents', and the school's answers frequently do not intersect, and that the failure of the plan is usually determined at that intersection.

The schedule typology was presented in the detail in which it is used, with the characteristic response patterns of the four schedules and the clinical objectives for which each is appropriate, and the two practical consequences were drawn out: that a small daily bonus is a fixed-ratio schedule and produces end-of-day bursts rather than continuous performance, and that a young person checked weekly will work at the end of

the week. The distinction between acquisition and maintenance schedules was developed, with the thinning rule and the transition to natural consequences, and the three forms of differential reinforcement were set out with the clinical argument for differential reinforcement of compliance, which is the technique that permits a practitioner to work on a problem behaviour while maintaining a positive relationship.

The management of escape-maintained behaviour was treated at length, on the grounds that it is the most practically important single category in applied practice. The circularity was described — the adult's response to the escalation is the maintaining contingency — and the five-step procedure was set out, and the systematic failure of punishment in this case was explained rather than merely reported: punishment teaches the young person that adults become larger, leaves the avoidance untouched, and does not teach compliance.

Contingency management was defined as the negotiated restructuring of a setting's reinforcement structure, and its eight-step procedure was specified, with the training of staff identified as the step most often omitted and most often responsible for programme failure. Behaviour contracts were described with the principle of reciprocity, and the three limits that apply to contracts with minors were stated: that the consequences must be within the adult's power to deliver, that no contract may trade a young person's rights, and that the contract must be genuinely voluntary.

The case of Ahmed illustrated the discipline's central discipline and its central danger. The functional analysis of his refusal to eat was elegant, and it was wrong, because the restriction had begun with an unexplained illness and the contingency analysis could not see it. The multi-disciplinary sequence, the medical assessment preceding the behavioural work, the graded hierarchy, the elimination of the "healthy lunch club" arrangement, the removal of the attention that the adults believed was encouraging, and the mother's anxiety as the true clinical task, are all described in the case.

The evidence is strong for parent training, for token economies in educational settings, for differential reinforcement, and above all for contingency management in substance use. The limitations are the dominance of the younger age range and of oppositional behaviour, the predominance of behavioural over experiential outcomes, the dependence on distinctive settings, and the substantial allegiance effects.

The ethical analysis addresses the market objection to reward-based practice seriously and derives four principles from it: never contract for what should not be traded, reinforce the behaviour rather than the practitioner's preferences, prefer natural consequences, and be honest about the effect of the system on the relationship. The cultural analysis addresses the cultural specificity of reinforcers, the family's existing systems of reward, the public humiliation that a visible system can produce, and the question of the professional's standing to set contingencies. The ethical requirements are listed in full, and the principle that a programme a young person cannot meet is a programme that must change rather than a young person who must be punished closes the section.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. State the three-term contingency and explain the role of each term.
2. Explain how the operant principle differs from the respondent principle, and give a clinical example of each.
3. Describe the three methods for establishing the reinforcing value of a stimulus, and explain why testing is the only decisive one.
4. Explain what a reinforcer survey is, and explain why the three sets of answers frequently fail to intersect.
5. State the four schedules of reinforcement and the response pattern characteristic of each.
6. Explain the post-reinforcement pause and the scallop, and state the clinical setting in which each is observed.
7. Explain why a small daily reward produces a burst of behaviour at the end of the day rather than continuous performance.
8. Distinguish the acquisition schedule from the maintenance schedule, and state the thinning rule.
9. Explain what happens when extinction is applied to a positively reinforced behaviour, and state the term for the temporary increase.
10. Distinguish differential reinforcement of other behaviour, of an incompatible behaviour, and of the target behaviour, and give a use for each.
11. State the six components of a shaping plan.
12. Explain what prompt dependence is, state the prompt hierarchy, and describe the fade plan.
13. Explain why the fade should be slower for a young person with a learning difficulty, and why the fade criteria should be set beforehand.
14. Describe the escalating pattern of escape-maintained behaviour, and explain the role of the adult's response in it.
15. State the five steps of the clinical procedure for escape-maintained behaviour.
16. Explain the predicted course of differential reinforcement of compliance, and why the practitioner should prepare the young person for it.
17. Explain systematically why punishment fails with escape-maintained behaviour.
18. State the eight steps of contingency management and identify the step most often omitted.
19. Explain the negotiated alternative and describe the balance a practitioner must strike in using it.
20. Describe the structure of a behaviour contract and explain the significance of reciprocity.
21. State the three limits that apply to a behaviour contract with a minor.

**Review and Discussion Questions (continued)**

22. State three circumstances in which a behaviour contract is appropriate and three in which it is not.
23. Describe the case of Ahmed and state both the methodological lesson and the clinical danger it illustrates.
24. Summarise the status of the evidence for parent training, token economies, differential reinforcement, and contingency management for substance use.
25. State the four ethical requirements that follow from the market objection to reward-based practice.
26. Explain the significance of the principle that a programme a young person cannot meet is a programme that must change.

## 16. Practice Activities

**Practice and Application**

### Activity 26.1 — The Schedule Audit (75 minutes, individual)

Take a real behaviour in a placement or in your own life and identify the actual schedule on which its consequences are delivered. Work out the fixed or variable ratio or interval, the typical magnitude of the consequence, and the delay between the behaviour and the consequence. Then redesign the schedule to produce the behaviour you actually want, and write down what the young person or other person would experience as different. The exercise teaches that most real-world problems with contingency management are problems with the schedule rather than with the reinforcer.

### Activity 26.2 — Reinforcer Survey with Triangulation (90 minutes, field-based)

Conduct a short reinforcer survey with a young person, a parent, and a teacher, using the same ten questions for each. Tabulate the three answers and identify the intersection and the divergences. Then test one candidate reinforcer by delivering it contingent on a specific behaviour and observing what happens. Most students find that the intersection is smaller than they expected, and that the divergences are the most informative part of the exercise.

### Activity 26.3 — Escape-Maintained Analysis (90 minutes, small group)

In groups of three, take a real behaviour that is described as "non-compliance" or "defiance" in a placement. Map the escalating sequence in detail: what does the adult say, what does the young person do, what does the adult do next, and what does the young person do then. Identify the point at which the demand is withdrawn and the consequence that produces. Then write the five-step plan for the case, with the differential reinforcement of compliance and the alternative behaviour specified. The most common finding is that the escalation is a well-rehearsed and effective request for help.

### Activity 26.4 — Writing a Behaviour Contract (75 minutes, individual)

Draft a behaviour contract for a realistic situation with a young person, with the adult also bound by specific promises. Specify the review date, the exceptions, and the consequences of each party's failure to comply. Then mark the draft, and identify the two places in it where you have contracted for something that should not be traded, and rewrite them. The second half of the exercise is the one that matters, and most drafts fail it in at least one place.

### Activity 26.5 — The Fade Plan (60 minutes, individual)

Take a prompting procedure you have used or observed and write a full fade plan: the prompt hierarchy from least to most intrusive, the number of trials at each level, the advance criterion, the step-back rule, and the schedule for the transfer of the prompt from the clinic to a school or home setting. Then write what you would do if the behaviour deteriorated at the third level. The transfer step is the one that most hand-written plans omit and the one that most often causes the treatment to fail.

## 17. Annotated Reading Pathways

**For the principles.** Read Skinner (1938) and then the Ferster and Skinner (1957) schedules volume in excerpt, and read the Milner (1953) laboratory chapters on reinforcement and extinction. The laboratory texts are worth the effort, because the clinical vocabulary was built in them and a student who has read them will find the clinical literature much easier.

**For the clinical craft.** Read the parent training literature, which is the most practically useful behavioural literature for work with young people, and read the parent guides that accompany it. Then read the applied texts on differential reinforcement, which contain the practical detail the theory does not.

**For the escape-maintained pattern.** Read the classic functional analysis of oppositional behaviour with a child and family, and then read the critical literature on the behavioural perspective with families, which is where the discipline's most serious self-criticism is to be found.

**For the evidence.** Read the Cochrane reviews of parent training and the meta-analyses of token economies in educational settings, and read the contingency management literature in substance use, which is the most methodologically mature part of the applied evidence base.

**For the ethics.** Read the ethical guidelines for the behavioural approaches and the literature on reward-based parenting, and read the critical accounts of the token economy in institutional settings. The historical material is the best preparation for the consent and documentation work in Part VIII.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Operant Conditioning and Contingency Management** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is reinforcement, schedules, and the deliberate shaping of behaviour in clinical and educational settings. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning,

language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Jonas is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Jonas's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Jonas explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Jonas chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The

practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Jonas has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Jonas can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.

- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Operant Conditioning and Contingency Management

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Operant Conditioning and Contingency Management** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Operant Conditioning and Contingency Management** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary

language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

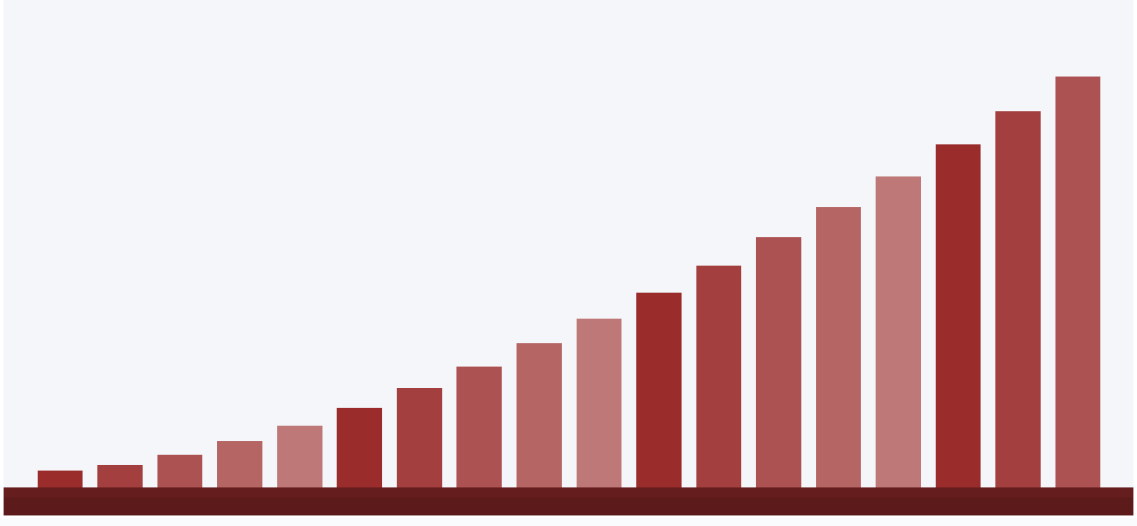
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Operant Conditioning and Contingency Management**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Operant Conditioning and Contingency Management to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 27

# Behavioral Activation and Activity Scheduling

The behavioural treatment of depression and withdrawal in adolescents.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the origins of behavioural activation in the treatment of depression, and state the theory of depression it is based on.
- Describe the three components of behavioural activation, and explain the mechanism of change each involves.
- Describe the treatment rationale and the course of behavioural activation with an adolescent.
- Explain the relationship between behavioural activation and the cognitive model, and between behavioural activation and the behavioural principles of Chapter 23.
- Apply activity monitoring and activity scheduling to a case of adolescent depression and withdrawal.
- Describe the evidence for behavioural activation with young people, and evaluate its limitations.
- Explain the adaptations required for young people, including the specific modifications for developmental level and for the comorbidity of anxiety.
- Critically evaluate the cultural and ethical dimensions of behavioural activation with young people.

## Chapter Overview

The cognitive revolution gave depression a theory of mind. Behavioural activation gave it a theory of action, and the two theories turned out to be compatible in a way that was not obvious at the time.

The behaviour therapy tradition had observed since the 1930s that the defining features of depression are not attributions and not beliefs but **withdrawal**: the reduction of activity, of engagement, of contact, and of reward. People who are depressed do not think differently about the world so much as they do *fewer things*, and the reduction in activity produces a reduction in reward, and the reduction in reward produces a further reduction in activity. The account is a behavioural account of a cyclical process, and it makes a prediction that the cognitive account did not: that interrupting the cycle would produce improvement, and that the improvement would precede rather than follow the change in thinking.

The intervention follows from the account. Depression is maintained by a deficit of rewarding activity and by the avoidance of situations that have been associated with non-reward. The treatment therefore consists of identifying the rewarding activities that the person has stopped, scheduling them, and doing them — and, where the activities have been stopped because they have become unrewarding, restructuring them so that they are rewarding again.

For adolescents, the case for behavioural activation is unusually strong, for reasons that are developmental. The adolescent's primary developmental task is the construction of a social identity through competence and connection, and withdrawal is directly opposed to that task. The adolescent with depression is not only distressed; the depression is interfering with the very process by which distress would ordinarily be resolved. The treatment that re-engages the adolescent in activity is a treatment that works on the depression and on the development at the same time, and this is one of the clearest cases in the book of a theory-driven intervention that addresses a developmental principle as well as a symptom.

This chapter presents the theory, the procedure, the modifications for young people, the evidence, and the case illustration.

# 1. Historical Context

## 1.1 The behaviour therapy tradition and depression

The first wave of behaviour therapy approached depression through the classical conditioning literature, and the account was of depression as a learned helplessness response to uncontrollable aversive events. Seligman's work on learned helplessness was the most influential formulation, and it drew on the animal literature on exposure to uncontrollable shock, in which the animal's failure to escape is learned and generalised to situations in which escape is possible.

The treatment implication was a form of exposure: the deliberate restoration of control. Seligman and colleagues developed what became known as the **control therapy** or the **mastery** approach, in which the patient is given tasks of graded difficulty whose successful completion demonstrates that outcomes are contingent on behaviour.

This work was important and it was largely eclipsed. The reason is partly historical, because the cognitive revolution captured the field, and partly conceptual, because the learned helplessness account of depression is a theory of the acquisition of a deficit and says little about how the deficit is maintained, which is the question that most depressed patients present.

## 1.2 The activity record

The most important antecedent to behavioural activation was Jacobson's work in the 1930s, and specifically the **activity record**: a daily log in which the patient recorded, in hourly units, the activities of the day and, for each, a rating of the degree of pleasure and of the degree of self-mastery achieved.

The activity record was developed for a purpose that the field has since forgotten. Jacobson used it to demonstrate that the depressed patient over-estimates the unpleasantness and under-estimates the pleasure of their activities, and that the discrepancy between the patient's report and the actual experience is itself a maintaining factor: the patient avoids the activity on the basis of an expectation that the activity is unpleasant, and the avoidance prevents the disconfirming experience. In this respect, the activity record was the first treatment technology to target a belief through behavioural data rather than through argument, and the discrepancy it measured is the ancestor of the modern behavioural activation rationale.

## 1.3 The cognitive revolution and the divergence

The cognitive model of depression, developed from Beck's work in the 1960s, proposed that depression is maintained by a specific pattern of negative automatic thoughts, by negative core beliefs, and by the cognitive triad. It produced an effective treatment, and for two decades cognitive therapy was the dominant approach.

The divergence between the two traditions was sharp in the 1970s and softened in the 1990s, and the softening came from a direction nobody expected: from the recognition, inside the cognitive tradition, that behavioural activation was effective and that its effect on mood was more rapid than that of the cognitive interventions. The treatment produced a substantial improvement in mood in the first few sessions, before any cognitive work had been done, and this was a striking finding because the cognitive model had predicted that the cognitive work should come first.

The resolution was the observation that the cognitive and the behavioural interventions address the same process at different points in the cycle, and that the behavioural intervention acts upstream, on the

behavioural withdrawal that generates the negative thoughts, and the cognitive intervention acts on the content of the thoughts. The two are complementary, and the combination — which is now the most common form of the treatment — is described in Chapter 16 and Chapter 17.

### 1.4 Behavioural activation as a distinct treatment

The 1970s and 1980s saw the development of behavioural activation as a treatment in its own right, most influentially by Jacobson and colleagues, and later by the group associated with the New Haven school, who produced the treatment manual and the outcome studies.

The rationale was stated in a form that has become the standard statement of the treatment:

> *Depression is a disorder of behaviour in which the person has withdrawn from rewarding activities, and the withdrawal is maintained by the avoidance of situations in which the person has experienced non-reward and by the loss of the reinforcing consequences of activity. The treatment consists of the identification of activities that the person finds rewarding, the scheduling of those activities, and the performance of them, together with the reduction of avoidance and the mastery of necessary tasks.*

Two features of this rationale are worth emphasising, because they distinguish behavioural activation from everything else in the book.

**It predicts the sequence of change.** Mood improves before cognition changes. This is a testable and unusual prediction, and it has been supported.

**It explains why the depression persists even when the person "knows" better.** The person with depression has usually tried to think their way out of it and failed, because the withdrawal continues, and the withdrawal is what maintains the negative thoughts.

## 2. The Theory

### 2.1 The behavioural model of depression

The behavioural model holds that depression is not primarily a disorder of mood with a disorder of thought attached. It is a **disorder of behaviour** in which four processes operate together.

**The reduction of reinforcing activity.** The person stops doing the things that are rewarding. The reasons are a lack of energy, an absence of expectation that the activity will be rewarding, and the withdrawal itself.

**The increase in aversive activity.** The person substitutes activities that are aversive or numbing: rumination, worry, self-criticism, substance use, sleep, screen time, and overeating. These activities are frequently more immediately reinforcing than the alternatives, which is why the depression is so difficult to escape by intention.

**The avoidance of situations associated with non-reward.** The person avoids situations in which they have experienced rejection, failure, or a lack of recognition. The avoidance prevents the corrective experience.

**The disruption of the circadian and the physiological systems.** Sleep, appetite, activity, and light exposure become irregular, and the irregularity is itself a maintaining factor.

The model is a cycle rather than a list, and the cycle is the treatment's target: the withdrawal reduces the reward, the reduced reward increases the withdrawal, and the increasing withdrawal increases the avoidance, which further reduces the reward.

### 2.2 The role of the cognition

The behavioural model does not exclude cognition; it locates it. Cognition enters at three points.

**As an anticipation.** The person does not undertake an activity because they expect no reward from it. The expectation is the mechanism of the avoidance, and it is directly testable against the actual outcome.

**As an attribution.** The person attributes outcomes to global and stable causes — "I never get anything right", "nothing ever works out for me" — and the attribution generalises from a specific event to the whole of the person's life.

**As a self-evaluation.** The person evaluates the self through the activities undertaken and the results obtained, so that the reduction in activity reduces the number of occasions on which the self can be evaluated positively.

The behavioural approach to each of these is the same: change the activity, and the cognition follows. This is why behavioural activation can be delivered without cognitive disputation, and why it is often the preferred treatment for a young person who is not ready to examine his own thoughts.

### 2.3 The reinforcing value of activity

The treatment rests on an assumption that is worth examining: that activity is reinforcing, and that the reinforcing value of an activity is a function of its *immediacy* and its *certainty*.

**Immediacy.** A consequence that is delayed is less reinforcing, and this is why many of the activities that a depressed adolescent gives up are those with delayed reward — studying for an exam, applying for a job, learning an instrument, writing to a friend. The behavioural approach is to restructure the activity so that the reward is immediate: study for twenty minutes and then do something enjoyable, rather than studying for three hours in the hope of a result in June.

**Certainty.** An activity whose outcome is uncertain is less reinforcing than one whose outcome is predictable, and this is the rationale for the graded task in mastery work: a task with a predictable and achievable outcome provides a reliable small reward at a time when the young person needs reliable small rewards.

**Magnitude.** A large activity is more rewarding when it is done, but it is much less likely to be done, and in a depressed adolescent the probability of undertaking matters more than the magnitude of the reward.

**Social contact.** Almost all activities that maintain well-being in adolescence are social, and the social component is usually the component that has been lost. The activity schedule must therefore be a schedule of *contact* as much as a schedule of tasks.

## 3. The Three Components

### 3.1 Activity monitoring

**Activity monitoring** is the daily record in which the young person logs their activities, together with a rating of the pleasure and of the mastery associated with each. The Jacobsonian version used hourly units across the day; the modern versions use a simpler daily or twice-daily log, and the simplification is a developmental adaptation.

The purposes of the monitoring are four.

**To establish the baseline.** The clinician needs to know what the young person's days actually contain, and the answer is frequently surprising. A young person who presents as "doing nothing" has days that are full — of sleeping, of scrolling, of eating, of avoidance, of small tasks that occupy the time without occupying the person.

**To make the discrepancy visible.** The monitoring shows, on the page, the difference between the activity and the predicted quality of the activity. The young person can see the entries for the Saturday when they "did nothing" and can see the rating of pleasure recorded against it.

**To establish the pattern.** A weekly graph of activity levels against mood levels shows the relationship, and it usually shows it clearly.

**To provide the data for the schedule.** The activities with the highest pleasure ratings and the lowest mood ratings are the candidates for the schedule, and this is why the monitoring precedes the scheduling.

**The practical problem with monitoring is compliance**, and the clinician should expect that a depressed adolescent will not complete the log. Three responses are useful. The log should be short. The clinician should complete the first day with the young person, sitting with them and logging their day. And the log should be reviewed in the session, not collected and left.

## 3.2 Activity scheduling

**Activity scheduling** is the specification of what the young person will do, when, and with whom, and it is the active component of the treatment. The specification should be written in three columns: the activity, the day and time, and the person involved.

Four principles govern the schedule.

**Schedule activities that are rewarding, in the sense of the section above.** The activity should be selected on the basis of the pleasure ratings from the monitoring, and it should be one the young person has actually enjoyed at some point, not an activity chosen by the clinician because it would be good for them.

**Schedule mastery activities alongside pleasure activities.** This is the Jacobson and colleagues' contribution and it is the part most often omitted. The depressed adolescent's problem is not only that they do nothing; it is that they have stopped doing the difficult things, and the difficult things are where the self-evaluation is generated. The mastery activity is specified in graded steps, with the smallest step achievable.

**Schedule social contact.** Because the social element is the one that has been lost and the one that most distinguishes the adolescent depression from the adult depression, the schedule must include contact with other people, and the contact must be specified in terms of the people and the duration.

**Schedule into the difficult slots.** The schedule is built around the times in the week when the withdrawal is worst, and not around the times when the young person is already functioning.

## 3.3 Activity monitoring and the graded task

**Graded task assignment (GTA)** is the third component, and it is the one most associated with behavioural activation and with the treatment of depression in young people.

The procedure has two halves. In the first, the young person and the clinician construct a **task hierarchy** of activities, from those that are currently impossible to those that are currently easy, with the activities graded by difficulty, and with the steps specified in the units in which they would actually be done. A typical hierarchy for an adolescent who has withdrawn from school might run: get out of bed before my mother calls me; get dressed; have breakfast; sit at the table with my phone; walk to the end of the road; walk to the shop; go to the college café; go to a lesson.

In the second, the young person works up the hierarchy, completing one or two steps beyond their current position, and each completed step is recorded and acknowledged, and the acknowledgement is the reinforcement.

The hierarchy is constructed collaboratively and is based on the young person's own specification, and the clinician's role is largely to break the steps down into a fine enough grain. The most common clinical problem is a hierarchy whose first steps are too large, and the fix is always the same: subdivide further, until the first step is something the young person is confident they could do today.

**The rules of graded task assignment** that the evidence supports are specific. The tasks should be achievable but not trivial. The young person should choose the tasks, from a list the clinician has helped to construct. The hierarchy should be reviewed and adjusted weekly. Completion should be recorded and acknowledged immediately. The tasks should be broken down until the first step is clearly achievable, and no exceptions to this rule should be made.

### 3.4 The session structure

The typical behavioural activation session is short, structured, and practical.

**Review of the monitoring.** What did the young person do, and what was the mood? The review is brief and the clinician is careful not to moralise about the content of the log.

**The discrepancy.** The comparison between the predicted and the actual quality of the activities. The clinician's posture is curious rather than triumphant.

**The schedule.** The activities for the coming week are agreed and written down, and the young person takes the schedule away, and the schedule is the homework.

**The graded task.** One step of the hierarchy is added, and the step is small.

**The problem-solving of obstacles.** What is going to make this hard, and what will you do? The question is asked in advance, because it is much easier to solve the problem on Wednesday than in the session on Friday.

**The social component.** Where the young person is isolated, the schedule includes the specific steps by which contact will be made, and these are graded like everything else.

## 4. Behavioural Activation with Children and Adolescents

### 4.1 The developmental case

The case for behavioural activation with adolescents is stronger than the case for adults, and the reasons are developmental.

**The withdrawal is developmentally costly.** The adolescent's task is the construction of a stable identity through competence and connection, and every week of withdrawal is a week in which the developmental work does not happen. A young person who withdraws at fourteen has a harder task at twenty.

**The adolescent's mood is more behaviourally determined.** The adolescent's emotional state is substantially a function of activity, sleep, and social contact, and the behavioural levers are correspondingly more powerful.

**The cognitive intervention is harder.** A young person with depression is often not able to examine his own thoughts productively, and may find the cognitive model alienating, and behavioural activation gives him a way of working that does not require him to challenge his own perceptions. Many adolescents who refuse cognitive therapy will accept behavioural activation.

**The competence dimension is directly available.** The graded task is a developmental intervention as well as a clinical one, because it generates evidence of competence, and evidence of competence is what the adolescent's self-evaluation requires.

**Social reconnection is a protective factor.** The literature on adolescent development identifies supportive social connection as one of the most robust predictors of good outcomes, and behavioural activation is one of the few treatments that targets it directly.

## 4.2 The specific modifications

**Shorten the monitoring.** A daily log with two ratings, completed in under three minutes, is realistic. The full hourly record is not.

**Complete the first day with the young person.** A depressed adolescent will not start the log unaided, and the session in which the clinician logs the young person's day alongside them is often the session that establishes the alliance.

**Use a social, concrete activity list.** The clinician proposes activities from a list the young person has helped to make, and the activities are named specifically — "walk to the bridge and back" rather than "exercise" — and the specificity is what makes them doable.

**Grade more finely than feels necessary.** The clinician will feel that the steps are too small. They are not.

**Prioritise contact over productivity.** For a withdrawn adolescent, a coffee with a friend is a more valuable activity than a fortnight of revision, and the schedule should say so.

**Build around existing anchors.** The days on which the young person does something already — a music lesson, a shift, a football match — are the days on which the rest of the schedule should be built, and the clinician should look for them in the monitoring.

**Involve the family carefully.** Family involvement supports the schedule and provides the practical conditions, and it also introduces the risk that the schedule becomes a family performance. The young person should be consulted about what is shared.

## 4.3 The management of comorbidity

**Anxiety.** A young person with depression and anxiety presents a specific difficulty: the activities that relieve the depression are the activities the anxiety prevents, and the activities the anxiety tolerates are the ones that do not relieve the depression. The resolution is graded exposure combined with activation, and the principles of Chapter 25 apply, with the added specification that the exposure and the activity are the same activity.

**Trauma.** A young person whose withdrawal is trauma-related is not helped by an activity schedule that pushes them into situations in which they feel unsafe, and the trauma assessment in Chapter 38 comes first. The activity schedule in that case is built inside the young person's window of tolerance, and Chapter 40 develops this.

**Substance use.** A young person who uses substances to manage the depression presents a situation in which the behavioural analysis must account for the function of the substance use, which is frequently relief from the very states the activity schedule is designed to address. Chapter 47 addresses this in detail.

**Apathy and the loss of interest as a symptom.** A young person with depression may not be able to identify any activity they find rewarding, and the monitor may record no pleasure at all. The clinical response is to start with activities that are *mastery* activities and that are physically activating, and to rely on the improvement in energy that follows.

### 4.4 The school context

Depressed adolescents are often not in school, and the school is therefore the setting in which the activation must be designed.

**Attendance as the first target.** A graded return to school, using the first-day design from Chapter 25, is often the first step in the graded task hierarchy, and it is frequently the graded task that has the greatest downstream effect, because attendance unlocks the rest of the school-based activity.

**The school-based activity schedule.** A short daily schedule of specific activities, with at least one social component, agreed with the school, is more effective than a schedule kept at home, because the young person is in the building anyway.

**Coordination with the pastoral team.** The pastoral team is usually already managing the young person's absences and is likely to be applying a monitoring system of its own. The behavioural activation schedule and the school's attendance system should be the same system, and a practitioner who adds a third system has increased the burden rather than reduced it.

## 5. Case Illustration: The Young Person Who Stopped Leaving Her Room

**Presentation.** Nadia is sixteen and has been out of school for seven weeks. She spends her days in her room, emerging to eat and to use the bathroom. Her mother reports that she used to be "loud and funny" and is now "like a ghost". She has not been in the same room as her younger sister for most of the period. Her mother is at work during the day, and the youngest sibling is at school, so the house is empty.

Nadia's own account, given with some reluctance at the fourth session, is that she stopped going in because "everyone's got their group and I'm not in one, and I'm not going to be the new girl again." She has not been to school in seven weeks and describes each morning as "wanting to disappear".

**The assessment.**

*The mood.* Nadia describes low mood, anhedonia, poor sleep, poor appetite, and a loss of energy. The picture is a depressive episode with a marked social withdrawal, and there is no history of mania, of psychosis, and of any previous episode.

*The functional analysis of the withdrawal.* This is where the behavioural work begins, and the analysis is short.

The antecedents of staying in bed are: waking, and the morning's requirement to get dressed and travel. The consequences are: the escape from the demand, and the relief. But there is a second and more important consequence, and the monitoring makes it visible. Nadia's mother, in desperation, brings her breakfast to the room, and stays and talks to her for twenty minutes, and does not make her come down. The mother's attention — which is warm, and is not given under any other circumstances — is contingent on the withdrawal. The mother is reinforcing the withdrawal with the very behaviour that is most sustaining of it, and she is doing it out of love.

The consequences of going to school, by contrast, are that Nadia spends a day in a building where she is not known, is looked at, and is required to interact. The school has been, in effect, delivering a strong punishment for attending.

*The antecedents of the social withdrawal.* Nadia's account identifies a specific trigger: a change of school in the previous year, and a period of a few weeks in which she was not spoken to by anyone. The withdrawal is not a mood that began the absence; it is a response to a specific social loss that was then maintained.

**The formulation.** A depressive episode, maintained by a behavioural cycle: the withdrawal reduces the reinforcing activity, the reduced activity produces a deeper depressive mood, the deeper mood produces a wider withdrawal, and the mother's daily attention to the room sustains the withdrawal behaviour itself. The social anxiety is real and is not a symptom of the depression; it preceded it, and it is the reason the depression is maintained.

**The intervention.** The plan has five elements, and the sequencing is critical.

*Element one: the mother's contingency.* The mother is asked to stop bringing breakfast to the room and to stop sitting with Nadia in the morning, and this is the hardest part of the plan. The reason is explained carefully: the daily attention is what is keeping the pattern in place, and the love that is in the attention is not in question. What is proposed instead is a daily contact that is not contingent on the withdrawal — a fixed fifteen minutes at a fixed time, with a specific activity, in which the mother talks about her own day, and which Nadia may or may not attend. The purpose is to preserve the relationship while removing the contingency.

The mother also agrees to a gradual reduction in the practical support: the breakfast comes to the kitchen, the door is left open, the answer to the question "is Nadia up?" becomes "she's in her room", and the mother's own daily functioning is protected, because a mother who has not slept properly for seven weeks cannot implement any plan.

*Element two: the graded task.* Nadia constructs the hierarchy, with the help of the counsellor, and it runs: get up; get dressed; come downstairs; have breakfast at the table; sit in the garden; walk to the corner; walk to the park; go to the shops; get the bus; go to college.

The first step — get up — is placed at the level where Nadia is confident, and the advance criterion is three consecutive days. The first four steps are all achieved within the first fortnight, and the counsellor does not

push past the fifth step even when Nadia is ready, because the advance criterion is a clinical tool and not a suggestion.

*Element three: the social contact.* The graded task alone produces an improvement in mood that is real but limited, because the tasks are solitary. The schedule therefore adds, in week two, one scheduled contact per day with a named person, and the contact is chosen with Nadia — a former friend, a cousin, a member of the football team Nadia used to play in — and each contact is preceded by a text and is short, and the shortness is the point, because the first successful contact after seven weeks of isolation is a short one.

*Element four: the school.* The graded return to school uses the first-day design. Nadia attends for one lesson in week three, in a subject she does not feel exposed in, with the pastoral support worker, and leaves having spoken to one person. The school plan specifies what will happen in the corridor, which is the place she most fears, and the plan has the pastoral support worker walk with her from the gate to the classroom for the first two weeks.

*Element five: the monitoring continues.* The daily log continues throughout, because it is the data that will show whether the plan is working, and because it is one of the few things in Nadia's life that is structured and predictable.

**The outcome.** At three months, Nadia is in school four days a week, taking three subjects, and is meeting one friend weekly. Her mother reports that the mornings are "normal-ish". The depression has not fully lifted and the work continues, and the partial response is the expected one: behavioural activation works faster on the behavioural symptoms than on the mood, and a young person who has withdrawn for seven weeks does not rebuild a social world in three months.

**The interpretation.** Two features of this case are generalisable. The first is the maternal contingency, which is present in a large proportion of cases of adolescent withdrawal, and which is rarely identified because it presents as care. The second is the ordering of the work: the graded task before the social contact, and the social contact before the school return, because the school return requires a social confidence that the graded task alone cannot supply. A practitioner who sent Nadia to school in week one would have produced a worse outcome than one who did nothing, because the failure would have confirmed her expectation.

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# 7. Evidence and Research Support

## 7.1 The status of the evidence

Behavioural activation has a larger evidence base than most psychotherapies, and the findings are unusually consistent. The principal results are as follows.

**Behavioural activation is effective for depression in adults.** The Cochrane and meta-analytic reviews report effect sizes in the large range, and the treatment is recommended in the major international guidelines.

**Behavioural activation is effective for depression in adolescents.** The adolescent literature is smaller but is consistent with the adult findings, and the reviews report moderate-to-large effects on both symptoms and functioning.

**Behavioural activation produces more rapid improvement in mood than cognitive therapy in the acute phase**, and this is the finding that established the treatment's viability as a standalone treatment and that motivated the combination with cognitive therapy.

**Behavioural activation is at least as effective as cognitive therapy on measures of depression severity**, and the two are not clearly different on the outcome measures, though the treatment acceptability data favour behavioural activation, particularly with young people and with people whose symptoms include a predominance of fatigue and anhedonia.

**The combination of behavioural activation and cognitive therapy is probably the most effective treatment for depression in adolescents**, and this is what most guidelines now recommend, with the behavioural component delivered first.

**Behavioural activation has good support in populations that are difficult to engage**, including young people who are reluctant to talk, and the treatment can be delivered by trained non-professionals with supervision, which is relevant to school-based and primary care services.

## 7.2 The component evidence

The treatment is composed of identifiable components, and the component studies are more useful for practice than the outcome studies.

**Activity monitoring** is the component with the weakest independent evidence, and this is a point of some controversy in the field. The counter-argument is that monitoring is not a treatment in itself but a mechanism that makes the discrepancy visible to the young person, and that its value is in what follows rather than in itself. The practical guidance from the evidence is to keep the monitoring brief and to do it collaboratively.

**Activity scheduling** has good support and is the component with the clearest mechanism.

**Graded task assignment** has good support, particularly with young people and in the treatment of depression with apathy, and the evidence for its use in activity scheduling for individuals with severe depression and low energy is reasonably strong.

**The discrepancy between the predicted and the actual quality of an activity** is a well-established finding, and the value of drawing it explicitly to the young person's attention is the therapeutic core of the treatment.

**The primacy of the behavioural component in the acute phase** is supported, and the treatment sequence is to activate first and to cognitively address the remaining difficulties once the behavioural cycle has been interrupted.

### 7.3 Limitations of the evidence

**The adolescent literature is smaller than the adult literature**, and the trials are heterogeneous in the age range, the severity, and the inclusion criteria.

**Long-term maintenance is less well studied than symptom reduction**, and a substantial proportion of young people relapse after the end of treatment, which is consistent with the developmental reality that the conditions that produced the depression may still be present.

**The trials have generally not included the most severely impaired young people**, particularly those with comorbid substance use, with trauma histories, or with neurodevelopmental conditions, and these are the young people most likely to be seen in a school or specialist service.

**The measures are largely self-report**, and the adolescent literature's reliance on self-report of mood is a limitation, particularly in a population whose reporting is affected by the very symptoms being measured.

**The allegiance effects are substantial** in this literature, as they are in the behavioural literature generally.

## 8. Strengths and Limitations of Behavioural Activation

### 8.1 Strengths

**It works quickly.** The behavioural component often produces a measurable improvement in mood within two weeks, and a rapid improvement is itself a therapeutic event, because it gives the young person evidence that the situation can change.

**It is concrete and accessible.** A young person who has been told that their thoughts are distorted usually responds poorly; a young person who is told that they will walk to the park and then come back and tell someone what they noticed is likely to go along with it.

**It is compatible with the developmental task.** The treatment works on the same processes that the adolescent's development requires, and this is a rare alignment.

**It is deliverable by non-specialists.** The treatment can be delivered by a school counsellor, a primary care nurse, or a trained support worker with supervision, and the evidence for the supervised delivery is good.

**It produces data.** The monitoring generates a record that the young person, the family, and the school can all see, and the record is itself a resource in the work with the school.

### 8.2 Limitations

**It is incomplete.** Behavioural activation addresses the behavioural withdrawal and its consequences. It does not address the reason the young person withdrew in the first place, except insofar as the withdrawal is maintained by a contingency, and the social anxiety in Nadia's case required work that the graded task alone would not have provided.

**It can be experienced as a productivity demand.** A young person who is told to "get up, get dressed, go to the park" may experience the schedule as another set of expectations, and the young person who is in the grip of a depressive episode may not be able to comply and will conclude that they have failed again. The clinical response is to make the first steps trivially small and to make the review of the log collaborative rather than evaluative.

**It does not reach the cognitions quickly enough in some cases.** A young person with a strong global negative self-belief may need the cognitive work sooner than the standard sequence allows, and the combination of behavioural activation and cognitive therapy addresses this.

**The maintenance is the problem.** A young person who leaves treatment with an improved mood and an unchanged set of conditions will relapse, and the relapse prevention work of the cognitive tradition should be built into the plan from the beginning.

**It can be misapplied as a compliance programme.** A graded task schedule in a school that is presented to the young person as a set of targets to be met, with consequences attached, is not behavioural activation; it is a compliance programme, and the evidence does not support it.

## 9. Cultural and Ethical Considerations

### 9.1 Culture, activity, and what counts as rewarding

The entire treatment depends on the assumption that the young person will find certain activities rewarding, and that assumption is culturally patterned in ways that require attention.

**What is rewarding is culturally specified.** Activities that are rewarding in one context are not in another: sport, music, religious or spiritual practice, family ceremony, food and cooking, language use, craft, and community activity all vary in their valuing. A clinician who proposes a graded task of "go to the library and read" to a young person for whom reading is associated with academic failure has proposed a task that is unlikely to be rewarding, and the failure will be read as motivation.

**Family obligation may be the available reward.** In some contexts the most reliably reinforcing activity available to a young person is the participation in family obligations and in the family's projects, and this may be the only socially legitimate way to build a schedule. A schedule built around helping at the family's shop, or caring for a grandparent, or the family's shared meals, may be both more effective and more

acceptable than one built around the activities a clinician would nominate.

**Gender expectations shape the available activities**, and a schedule that puts a young woman into activities that expose her to unwanted attention in the community may be actively harmful, and the clinician should ask.

**The stigma of mental illness affects the schedule.** In some settings, a schedule that is visible to the family or the school and that names the young person's difficulties is harmful, and the schedule should be designed to be usable without being explanatory.

## 9.2 The relationship to the body and to appearance

Behavioural activation with adolescents inevitably touches the body, because the activities that relieve depression include physical activity, and because the young person with depression has often stopped eating, exercising, and attending to their appearance. Three cautions follow.

**The activation of physical activity can be experienced as a demand on the body**, particularly for a young person who has experienced body scrutiny, weight commentary, or disordered eating. The activities should be chosen with that in mind.

**Weight gain and appetite disturbance are features of the disorder**, and a young person who is told to eat more, or to exercise, may experience the instruction as an accusation. The clinician should name the symptom as a symptom.

**Anorexia nervosa is not treated by behavioural activation.** The graded task schedule is contraindicated in a young person with a diagnosed eating disorder, and the medical and specialist pathway in Chapter 46 takes precedence. This is an important exception and the practitioner should know it.

## 9.3 Ethical considerations

**Voluntariness.** The activities in a behavioural activation schedule are proposed, not imposed, and the young person chooses from a list. A schedule of activities chosen by the clinician is a different intervention with weaker support and a different ethical character.

**The monitoring and the surveillance boundary.** The daily log is a monitoring device, and the young person should know exactly what it is for, who will see it, and what will be done with it. A log that is shared with the school or the parents without the young person's knowledge is a breach of trust that will damage the alliance and is likely to end the treatment.

**Avoiding the allocation of tasks that should not be allocated.** A graded task hierarchy must not include tasks that the young person is not developmentally able to perform, must not include tasks that function as unpaid labour, and must not include the performance of household duties that other family members should carry.

**The graded task as a performance of recovery.** When the young person's graded task is shared with the family, it can become a public display of progress, which is a burden and can become a source of shame if the progress is slow. The clinician should keep the hierarchy private unless the young person wants it shared.

**The competence dimension and the risk of discouragement.** The graded task generates evidence of competence, and it also generates repeated evidence of failure if the steps are mis-set. The first step must be achievable, and the review must be framed so that a step that is not completed is information about the step

rather than about the young person.

**Coordination with the school must be consented to.** A practitioner who shares a young person's monitoring data with the school without the young person's knowledge has used the data for a purpose other than the one for which they were collected.

## 10. Key Terms

**Key Terms at a Glance**

- **Behavioural activation** — A treatment for depression in which the reduction of reinforcing activity and the increase of aversive activity are treated as the maintaining cycle, and the intervention consists of scheduling and performing rewarding and mastery activities.
- **Activity record** — Jacobson's daily log of activities with ratings of pleasure and of mastery, used to establish the baseline and to make the discrepancy between predicted and actual experience visible.
- **Activity monitoring** — The modern, abbreviated form of the activity record, used as a baseline, as a measure of the relationship between activity and mood, and as the source of the schedule.
- **Activity scheduling** — The specification of what the young person will do, when, and with whom, on a written schedule in three columns.
- **Graded task assignment (GTA)** — A hierarchical list of tasks of increasing difficulty, worked through one or two steps beyond the young person's current position, with each completed step recorded and acknowledged.
- **Task hierarchy** — The ordered list of graded tasks in a graded task assignment, running from currently impossible to currently easy.
- **Advance criterion** — The specified condition for moving to the next step of a graded task hierarchy, usually a number of consecutive completions.
- **Mastery activity** — An activity selected for the sense of competence it produces rather than for its immediate pleasure.
- **Pleasure activity** — An activity selected for the immediate reward it produces.
- **The discrepancy** — The difference between the expected quality of an activity and its observed quality; the therapeutic target of the treatment.
- **Behavioural model of depression** — The account in which depression is maintained by the reduction of reinforcing activity, the increase of aversive activity, the avoidance of non-reward situations, and the disruption of physiological rhythms.
- **Immediacy of reward** — The property of an activity's consequence that determines its reinforcing strength, and the basis for restructuring delayed-reward activities.
- **Certainty of reward** — The predictability of an activity's outcome, which determines its reinforcing strength and the rationale for graded mastery tasks.
- **Social contact in the schedule** — The deliberate inclusion of scheduled contact with other people, which is the component most specific to adolescent depression.

**Key Terms at a Glance (continued)**

- **First-day design for school return** — The practice of beginning a return to school with a single brief, structured, and rewarding visit, before any substantial assessment or homework.
- **Anhedonia** — The loss of the capacity to experience pleasure from activities that would ordinarily be rewarding.
- **Behavioural activation with cognitive therapy** — The combination in which the behavioural component is delivered first, to interrupt the cycle, and the cognitive component follows.
- **Control therapy** — The early behavioural approach to depression based on the restoration of control through graded mastery tasks.
- **Learned helplessness** — The account of the acquisition of a passivity response through exposure to uncontrollable aversive events, with limited application to adolescent depression.
- **Pacing** — The deliberate matching of the task difficulty to the young person's current capacity, with the expectation of variable performance.
- **Behavioural activation for apathy** — The application of activity scheduling and graded tasks to the predominance of fatigue and loss of interest in the depressive presentation.
- **Window of tolerance** — The range of arousal in which a person can function and process information, and outside which behaviour is dysregulated; the constraint within which an activity schedule must be built.
- **Contingency management of the depressive cycle** — The application of the operant analysis to the relationship between the young person's activity, their mood, and the contingencies that sustain the withdrawal.
- **Activation of the social self** — The reintroduction of contact with other people, graded, as the mechanism by which the depressive cycle is interrupted in the adolescent presentation.

## 11. Chapter Summary

Behavioural activation is the treatment that the behavioural tradition contributed to the understanding and treatment of depression, and it rests on a single observation that is older than the cognitive model and, in some ways, more useful: the defining features of depression are not beliefs but withdrawal, and the withdrawal is self-maintaining.

The behavioural model describes depression as a cycle with four processes: the reduction of reinforcing activity, the increase of aversive and numbing activity, the avoidance of situations associated with non-reward, and the disruption of the physiological rhythms. The intervention interrupts the cycle by identifying activities that the person finds rewarding, scheduling them, and performing them, and by adding graded mastery tasks where the young person has stopped doing the difficult things.

The three components were described in detail. Activity monitoring establishes the baseline, makes the discrepancy visible, identifies the pattern, and supplies the data for the schedule, and the chapter noted the three practical responses to the predictable non-compliance with it. Activity scheduling requires the specification of what, when, and with whom, and it must include social contact, because the social

component is the one that distinguishes the adolescent depression from the adult depression and the one that has been lost. Graded task assignment is the component most associated with the treatment, and its rules — achievable but not trivial, chosen by the young person, reviewed weekly, subdivided until the first step is clearly achievable — were set out in full.

The case of Nadia showed the treatment in operation and showed the three elements of the formulation that generalise: the maternal contingency, in which the daily attention of a loving parent was sustaining the withdrawal; the ordering of the work, with the graded task preceding the social contact and the social contact preceding the school return, because a premature return to school would have confirmed the young person's expectation; and the distinction between the depression, which was the presenting problem, and the social anxiety, which preceded it and maintained it.

The evidence is large and consistent for adults and substantial and growing for adolescents, and the findings that behavioural activation produces more rapid mood improvement than cognitive therapy in the acute phase, and that the combination is probably the best available treatment for adolescent depression, are the ones that changed practice. The component evidence favours activity scheduling and graded task assignment, and the chapter noted the controversy over the monitoring component honestly.

The limitations are that the treatment is incomplete, that it can be experienced as a productivity demand, and that maintenance is the central problem, since a young person who leaves treatment with an improved mood and unchanged conditions will relapse. It can also be misapplied as a compliance programme, which is not the same intervention and is not supported by the evidence.

The cultural analysis addressed the specification of what is rewarding, the role of family obligation as the most reliably available reward in some settings, gender expectations, and the stigma of mental illness in the design of a schedule. The ethical analysis addressed the voluntariness of the activity selection, the surveillance boundary around the monitoring, the body and appearance issues that are unavoidable with adolescents, the specific contraindication of graded task scheduling in a young person with a diagnosed eating disorder, and the ethical requirements of keeping the hierarchy private unless the young person wishes otherwise.

## 12. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the historical development of behavioural activation from Jacobson's activity record to the modern treatment.
2. Explain the contribution of the activity record to the treatment and state its two principal purposes.
3. State the four processes of the behavioural model of depression.
4. Explain how cognition enters the behavioural model, and at what three points.
5. Explain the significance of the immediacy and certainty of a reward for the design of an activity schedule.
6. State the three components of behavioural activation and the function of each.
7. State the four purposes of activity monitoring.
8. Explain the three responses to the predictable non-compliance with monitoring.
9. State the four principles that govern activity scheduling, and explain the social contact principle.
10. Describe graded task assignment, and state its six rules.
11. Describe the session structure of behavioural activation.
12. State the five developmental reasons why the case for behavioural activation is stronger with adolescents than with adults.
13. State the six specific modifications required for young people.
14. Explain the difficulty of the comorbid anxiety presentation and the resolution.
15. Explain the role of attendance as a graded task in the school context.
16. Describe the case of Nadia, and state the three features of the formulation that generalise.
17. State the four elements of Nadia's intervention and explain the ordering.
18. Summarise the status of the evidence for behavioural activation with adults and with adolescents.
19. Summarise the component evidence, and state the controversy over the monitoring component.
20. State the five limitations of the evidence base.
21. Explain the four strengths and the five limitations of the treatment.
22. Explain how the assumption that certain activities are rewarding is culturally patterned.
23. Explain the significance of family obligation as an available reward in some settings.
24. State the three cautions relating to the body and to appearance, and the specific contraindication of the treatment in a diagnosed eating disorder.
25. State the five ethical requirements for behavioural activation with young people.

## 13. Practice Activities

**Practice and Application**

### Activity 27.1 — Your Own Activity Record (three days, individual)

Complete a two-column activity record for three days, with a pleasure rating and a mastery rating against each activity. Do not complete it retrospectively. On the fourth day, plot the mood rating against the activity and identify the relationship. Then write two sentences describing a discrepancy you found — something you expected to be unpleasant that was not, or something you expected to be pleasant that was not. The exercise is the treatment in miniature, and every student finds at least one discrepancy they had not noticed.

### Activity 27.2 — Building a Graded Task Hierarchy (75 minutes, individual)

Choose a young person from placement who is withdrawn. Construct a graded task hierarchy of at least twelve steps, running from currently impossible to currently easy, with the steps specified in the units in which the young person would actually do them. Then check the first step against the criterion: is it something the young person is confident they could do today? If not, subdivide and repeat. Most hierarchies fail this test at the third or fourth step, and the exercise is designed to make that visible.

### Activity 27.3 — The Social Contact Ladder (60 minutes, small group)

In groups of three, design the social reintroduction component of a behavioural activation plan for a withdrawn sixteen-year-old. Specify a graded ladder of contacts, the people involved, the duration of each, the preparation for each, and what is done if the young person refuses. Then discuss what the group would do if the young person said she did not want to see any of the people the group had chosen. The last part is the most instructive, because the answer is almost always that the young person chooses a different person, and the reason she could not choose one initially is the point.

### Activity 27.4 — Auditing a Schedule for Coercion (45 minutes, individual)

Take any activity schedule you have seen or designed, and audit it against five criteria: was the activity chosen by the young person; is the monitoring private; is the graded task free of tasks that should be allocated to others; is the schedule free of tasks that are unsafe for the young person; and is the first step achievable today? Count the failures. The exercise is a short version of the ethical analysis, and the failures are usually found in the monitoring rather than in the schedule.

## 14. Annotated Reading Pathways

**For the treatment.** Read Jacobson (1971) for the original account and Jacobson and colleagues (2001) for the controlled comparison with cognitive therapy, and read Rehm (2011) for the modern synthesis. The Jacobson material is old and the concepts are current, which is a rare and useful combination.

**For the evidence.** Read the adolescent review literature on behavioural activation and read the meta-analyses of the adult literature, and then read a study with a component analysis, which is more useful for practice than the outcome trials.

**For the developmental adaptation.** Read the adolescent development literature on the role of competence and connection, and read the treatment adaptation papers for young people. The developmental rationale is what makes the modifications in Section 4 principled rather than ad hoc.

**For the combination with cognitive therapy.** Read the cognitive model chapter in this book alongside the activation material, and read the studies of the combination, which is the treatment most likely to be used in practice.

**For the cautions.** Read the eating disorder and adolescent depression literature, particularly on the contraindication of behavioural activation in anorexia nervosa, and read the literature on activity monitoring as a burden.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Behavioral Activation and Activity Scheduling** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the behavioural treatment of depression and withdrawal in adolescents. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Priya is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Priya's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Priya explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Priya chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Priya has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Priya can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Behavioral Activation and Activity Scheduling

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Behavioral Activation and Activity Scheduling** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Behavioral Activation and Activity Scheduling** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Behavioral Activation and Activity Scheduling**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into

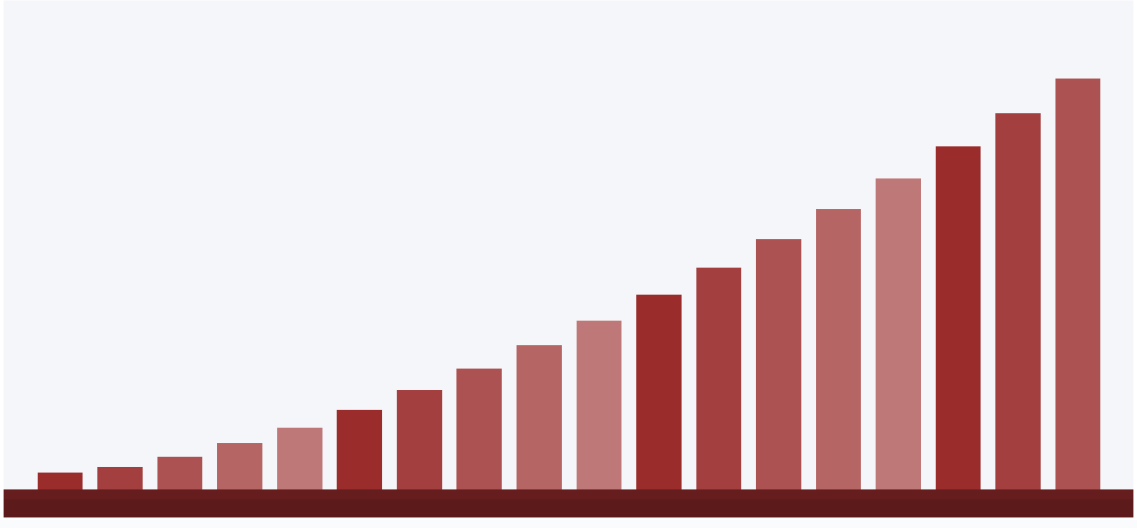
clinical governance.

**Written consolidation.** In 250 words, explain Behavioral Activation and Activity Scheduling to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 28

# Contingency Management in Schools and Clinics

Token systems, behaviour contracts, and the negotiated restructuring of reinforcement in real settings.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the rationale for using contingency management in schools and clinics, and state the conditions under which it is appropriate.
- Describe the design and implementation of a token economy in a classroom setting.
- Explain the design of a behaviour support plan, and describe the elements it requires.
- Describe the construction of a behaviour contract, and state its limits in the school context.
- Explain the role of the classroom teacher, the teaching assistant, and the parent in a contingency management programme, and describe the training they require.
- Describe the management of a classroom in which the behaviour of a small group is disrupting the class.
- Apply contingency management to a case in a school or clinic, and evaluate the ethical constraints.
- Critically evaluate the cultural and ethical dimensions of contingency management in institutional settings, and state the legal and regulatory context.

## Chapter Overview

Contingency management is the most direct application of the operant principles of Chapter 26 to a real setting, and it is also the application with the largest gap between its technical simplicity and its practical difficulty. The technique is easy to describe: define the target behaviours, define the consequences, make the consequences contingent, and review. The difficulty lies in the fact that a school is an organisation of many people, each of whom responds to a young person's behaviour, and a contingency system is only as good as the weakest of those responses.

The failure mode is predictable and it is the central clinical problem of this chapter. A programme is designed with the class teacher. The teaching assistant is not told. The dinner supervisor is not told. The supply teacher, who covers on the two days a week the class teacher is absent, is not told. On the first day the supply teacher sees a pupil being disruptive and responds with the customary escalation, and the pupil learns that the classroom contingency holds on Tuesday and Thursday and not on Wednesday. Two weeks later the data look like a mystery, and the practitioner must decide whether the hypothesis was wrong or the implementation was inconsistent, and the honest answer is usually the second and the diagnosis is usually harder than it should have been.

The other central theme is the status of the system within the setting. A token economy introduces an explicit exchange relation between the adult and the young person, and in a school that exchange is visible to peers. This is not a minor consideration: for a young person who is already struggling with status, a visible system of rewards and penalties can be more damaging than the behaviour it addresses. The practitioner must decide, in each case, whether the system is visible, and the decision is not always "no".

This chapter presents the technical design of school and clinic contingency systems, the implementation requirements that make them work, the failure modes, and the ethical and legal framework that applies.

## 1. The Setting as a Contingency System

## 1.1 Every setting is already a contingency system

The single most useful clinical idea in this chapter is that a school, a clinic, and a family are already systems of contingencies, operating whether or not anyone has designed them. The question is never whether to introduce consequences into a setting; the setting is already full of them. The question is only whether the consequences are the ones that serve the young person.

The inventory is short and it is diagnostic. For any behaviour that concerns a practitioner, the inventory asks: what happens immediately after it, for the young person, for the peers, and for the adult; what happens immediately before it; who is present; and what is the young person's alternative.

Three findings are common.

**The problem behaviour is reinforced.** The pupil who is sent out of class gets to leave a boring lesson; the pupil who is shouted at gets attention; the young person who refuses to attend gets to remain with the family. These are the classic patterns and they are everywhere.

**The desired behaviour is unreinforced.** The pupil who works quietly for forty minutes receives nothing; the pupil who asks for help is sent away; the young person who attends on time and quietly is invisible.

**The adults are inconsistent.** Two teachers in the same corridor respond to the same behaviour differently, and the young person has learned that the inconsistency is exploitable.

## 1.2 Why schools and clinics are the right sites for this work

Three features make schools and clinics the natural settings for contingency management.

**The contingencies are there.** The school's own organisation produces the reinforcements and the punishments; the practitioner does not have to build a system from nothing, only to change the existing one.

**The consequences are legitimate.** A school can legitimately offer a young person access to a preferred activity, a place on a team, a reduced homework load for a defined period, a role in a responsibility group. These are things the school controls and things the young person values.

**The setting is where the behaviour happens.** A behaviour changed in the clinic and not in the corridor has not been changed, and the school is the only place where the corridor behaviour can be observed and modified.

## 1.3 When contingency management is appropriate

Contingency management is appropriate when the following conditions are met, and the practitioner should be able to state the reasoning for the decision in each case.

**The behaviour is clearly defined and is a problem in itself.** The target must be observable and countable, and the case for changing it must be made in terms of the young person's or the setting's functioning, not in terms of the adults' convenience.

**The behaviour has been assessed.** A contingency designed on the basis of a guess is a guess with consequences attached, and a consequence that is wrongly specified is worse than no consequence, because it teaches the young person that the system's rules are arbitrary.

**The consequences are within the organisation's power to deliver.** A school cannot deliver a consequence that depends on another agency, and a clinic cannot deliver a consequence that depends on the family, unless the family is in the plan.

**The young person can succeed.** A contingency that the young person cannot meet has failed, and the response to failure is to adjust the system, not to increase the stakes.

**There is a plan for the transition.** A contingency system that is introduced and never withdrawn becomes a permanent dependency, and a young person's dependence on a system of external rewards is a developmental regression.

**The young person, or the group, has been consulted.** A system imposed on a group of students will be resisted, and the resistance will be rational.

### 1.4 When it is not appropriate

Contingency management should not be used where the behaviour cannot ethically be shaped, where the young person lacks the capacity to understand the system, where the consequence would be a deprivation of a right, or where the system's real function is to control rather than to teach.

A special caution is required in the case of young people with severe cognitive impairment. A token economy is dependent on the young person's understanding of the exchange, and a young person who does not understand it will respond to it randomly. For these young people, the contingencies must be direct and immediate and the system must be supported by the adults, and the plan for fading must be correspondingly slower.

## 2. The Token Economy

### 2.1 The design

A **token economy** in a classroom is a system in which specified behaviours are reinforced with tokens, which are exchangeable at a specified time for specified backup reinforcers. The design requires six decisions, and each of them is a decision that is usually made implicitly and should be made explicitly.

**The target behaviours.** A small number, clearly defined, observable, and countable, and chosen because their change matters. In a class of twenty-five with three pupils who are disruptive, the targets might be: hands up before speaking, remaining seated during the lesson, and completing the first three questions of the starter task. Six targets would be unworkable.

**The reinforcer.** The token itself, or an immediate item. The tokens should be small, durable, collectable, and immediately delivered. The backing reinforcers should be chosen by the young people and should be realistically obtainable.

**The schedule.** The density of reinforcement determines the behaviour. A dense schedule produces a rapid establishment; a sparse schedule produces a generalisation failure. The thinning plan should be written down from the beginning.

**The exchange procedure.** When, where, and by whom the tokens are exchanged. The exchange should be frequent, should be a positive event, and should not be a source of administrative friction for the staff.

**The response cost.** Whether tokens are lost for specified behaviours, and what the loss procedure is. The response cost should be small, immediate, and never a deprivation of a right.

**The fade plan.** The schedule on which the system will be withdrawn, and the criteria for withdrawal. A token economy with no fade plan is a permanent system.

## 2.2 The implementation

The implementation requirements are the part that determines success, and they are the part most often under-specified.

**Staff training.** Everyone who will deliver a token must be trained. The training should include, at a minimum: what the target behaviours are; what to do when they occur and what not to do; what to say and what not to say; how to deliver the token promptly and neutrally; and what to do in the situations that the training predicts will be difficult.

**A rehearsal.** The training should include a rehearsal, in which the staff practise delivering tokens in the presence of a simulated disruption, and in which the appropriate responses to the difficult cases are agreed.

**A written protocol.** The protocol should be on the desk, in the classroom, in the language of the setting, with the target behaviours, the responses, and the exception procedures. A protocol in a file in the office is not used.

**A monitoring system.** A record of the tokens delivered, which is both the fidelity check on the implementation and the data on the behaviour.

**A weekly review.** The review examines the data, the implementation fidelity, and the young people's own views, and it makes the adjustments.

## 2.3 Group versus individual systems

A class-wide token economy is a different instrument from an individual one, and the choice should be deliberate.

**The class-wide system** treats the class as a unit, with the consequences delivered to the class. It is simpler to administer, it does not single out any young person, and it has a significant additional effect: it changes the peers' behaviour towards each other, because a pupil who does not want to lose the class's tokens will manage a disruptive pupil in ways the teacher cannot.

The class-wide system has three limitations. It cannot address an individual young person's specific behaviour, because the consequences are too coarse. It risks the competitive dynamic in which a strong pupil dominates the token economy. And the target behaviours must be ones the whole class can achieve, which constrains them.

**The individual system** is more precise, more informative, and more visible to peers, and it is the appropriate choice for a young person working on a specific behaviour.

**The hybrid** is usually the best: a class-wide system for the shared classroom behaviours, and an individual system for a named young person working on a specific goal, with the individual system structured so that it is not publicly displayed.

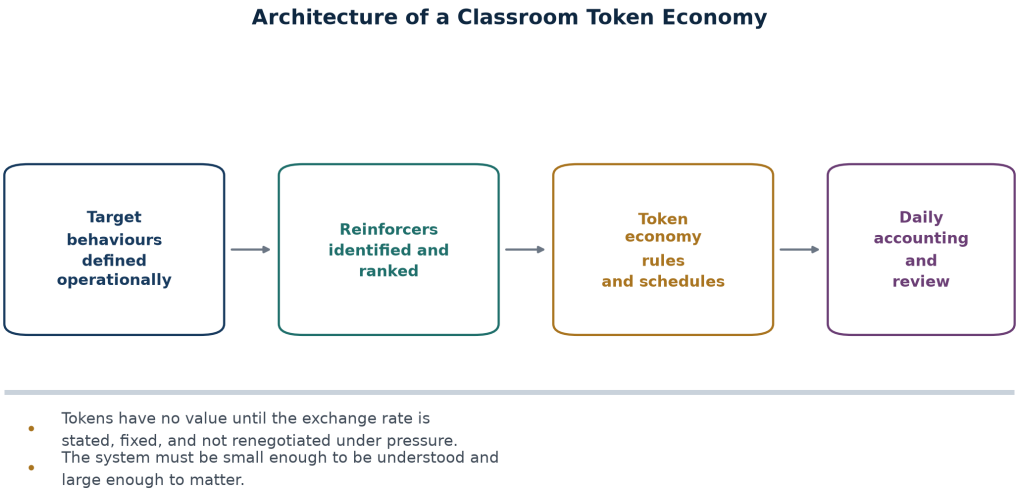



**Figure 28.16** A classroom token economy showing the target behaviours, the tokens awarded, the exchange table, the response cost rules, and the weekly review cycle.

### 2.4 The problems specific to token economies

**The contamination of the target behaviour.** Tokens become a currency, and young people learn to produce the behaviour for the token and to stop when the token stops. The fade plan is the only answer.

**The peer problem.** A visible token system makes some young people targets, and a young person with a history of being bullied may be in greater danger with tokens than without.

**The staff problem.** Token delivery is additional work, and the adults who do it least are usually those with the greatest need, and the result is an inconsistent system.

**The dismissal problem.** A pupil who is sent out of the classroom loses the opportunity to earn tokens, and the removal from the class is also a removal from the reinforcement system, which is why a token economy and an exclusionary response are incompatible in the same plan.

**The reward problem.** The backup reinforcers must be genuinely valued and genuinely available, and a school that promises a reward it cannot deliver, or that offers only what the young person considers worthless, has damaged the system rather than established it.

## 3. Behaviour Support Plans

### 3.1 The function

A **behaviour support plan** is the written document that specifies, for a named young person, the target behaviours, the strategies to be used, the contingencies, the roles of the adults, the data to be collected, and the review arrangements. It is the formalisation of contingency management for an individual, and it is the document that most schools lack and most need.

The function of the plan is partly clinical and partly organisational. Clinically, it ensures that the adults are working from a common specification rather than from individual impressions. Organisationally, it is the document that the practitioner can take to a meeting, and that survives a change of staff, and that can be reviewed.

## 3.2 The elements

**Student and staff information.** The young person's strengths, interests, and preferences; the adults involved and their roles; the timetable of the adults, so that the plan can be matched to the staffing.

**Target behaviours.** Stated in operational form, with the baseline data, and with the target specified in a way that is measurable.

**Strategies.** The specific techniques: differential reinforcement, planned ignoring, a specified response to disruption, a break procedure, a de-escalation script. The strategies should be specific enough to be followed and few enough to be remembered.

**The contingencies.** The rewards and the response costs, with the schedule.

**The response to escalation.** A specified, graduated, and rehearsed procedure, with the point at which the young person is removed from the setting and what is said when that happens. This element is the one most often missing and the one most responsible for failure, because when a young person escalates, the adult's improvised response is the treatment.

**Data collection.** What is recorded, by whom, and how the data are reviewed.

**A review schedule**, and the criteria for ending the plan.

**The involvement of the young person.** The young person's own words, their own goals, and their agreement, should be in the plan.

## 3.3 The graduated response and the escalation procedure

The escalation procedure should be specified in advance, in writing, and be common to all the adults, and it should include a clear statement that the removal from the class is itself a consequence and that the return is the reward. The commonest errors are to remove without a specified return condition, to remove with a variable duration, and to allow the young person to return as soon as they are calm, which reinforces the escalation, because the fastest route back is the loudest.

A better procedure specifies that the return is conditional on the completion of a defined task, and that the duration of the removal is short and fixed, and that the young person is accompanied on the return.

## 3.4 The behaviour support plan and the law

A behaviour support plan in a school is a document with legal and regulatory implications, and the practitioner must know the jurisdiction. In the United States, the framework is the Individuals with Disabilities Education Act, under which a behaviour that results in a change of placement is a manifestation of a disability, under the discipline procedures of IDEA, and under Section 504 of the Rehabilitation Act the protections apply to students with disabilities in any setting. The protections include the requirement of a functional behavioural assessment before a change of placement for a student with a disability, and the requirement that the plan be developed by a team.

A practitioner in a school who writes a behaviour support plan without checking the regulatory context is taking a legal risk as well as a clinical one, and the check takes ten minutes. The equivalent framework in the United Kingdom is the statutory framework of behaviour support and, for example, the Equality Act 2010 and the Children and Families Act 2014; elsewhere the practitioner must identify the local equivalent.

The principle is the same everywhere: the plan for a young person with a disability or with a special educational need is a document with legal standing and not merely a clinical convenience.

## 4. Behaviour Contracts in the School Context

### 4.1 The form

A school behaviour contract is a written agreement between a young person and the school, or between a young person and a class, in which specific behaviours are specified in return for specific consequences. The most useful form in the secondary school is the **class contract**, in which the class as a group agrees to a small set of behaviours in return for a set of privileges, and in which the young people have participated in drafting it.

The advantages of the class contract over the class-wide token economy are that it requires no currency, no counting, and no exchange procedure, and that it is far more visible and comprehensible to parents and to a school board. Its disadvantages are that it is less precise, that it is harder to modify mid-term, and that it has no natural fading mechanism — a class contract is typically a term-long arrangement, and the natural conclusion of it is its renewal, which is a decision that has to be made deliberately.

### 4.2 The limits

A behaviour contract in a school must not contract for the removal of a young person's rights: not for access to education, not for access to meals, not for access to the toilet, not for the removal of a statutory exclusion, and not for the removal of access to a member of staff who is the young person's advocate. A contract that trades an educational entitlement for compliance is unlawful in most jurisdictions, and a contract that trades it for a privilege is likely to be so.

A contract must also be genuinely voluntary, which in a school means that the young person can decline to sign without an academic or behavioural consequence, and the practitioner should be prepared for the case where the young person declines and the school wants to know what to do. The answer is to use the class contract, which involves the young person as a member of a group rather than as an individual signatory.

## 5. Case Illustration: The Class That Had Stopped Listening

**Presentation.** Mr Adeyemi teaches a Year 8 class of twenty-eight in a large secondary school. The referral from the pastoral team says that the class is "out of control" and that three students are responsible for the disruption. Mr Adeyemi has been teaching for twenty-two years and is close to the end of his tether. The head of year wants the class split.

**The assessment.** The practitioner asks for the data before the plan.

The class's behaviour log shows something that the referral has not. The disruption is concentrated in a single lesson, the last lesson on a Tuesday afternoon, and it is concentrated in a specific thirty-minute activity: the teacher setting a written task with a thirty-minute independent working period. The lesson before it — the same subject, the same class, the same teacher, an hour earlier — has a behaviour log with almost no entries. The difference is the activity, not the class, and the difference is not the teacher.

The practitioner observes the lesson. The pattern is clear. At the start of the independent working period, Mr Adeyemi sets the task and then circulates to the students who are stuck. This leaves the students who are working unattended, and three of them — the ones the referral names — start talking. The noise rises. Mr Adeyemi returns to the front and says, in a voice the class has heard a thousand times, "Some people are finding this really hard." The class knows who he means. The three boys laugh, because the statement is a ritual and not an instruction, and the two students nearest them join in, and the class as a whole spends the last fifteen minutes of the lesson in a low-level state of disruption that no single student could be held responsible for.

**The functional analysis.** This is a classroom-level functional analysis, and it is the analogue of the individual analysis of Chapter 24.

*Antecedent.* An unsupervised independent working period, and the teacher's attention leaving the class.

*Behaviour.* Talking, escalating to noise, occasional refusal to work.

*Consequence.* A ritualised, non-specific warning from the teacher, followed by the teacher's return to the front, followed by the class's attention, which is entertaining. The behaviour reliably produces a change in the adult's activity and a change in the attention of the class, and it does so whether or not it produces any work.

The behaviour is maintained by **peer attention**, and the adults' ritualised response, which is intended as a signal of disapproval, is a form of attention that the class reads as a signal of significance. This is the most common classroom dynamic the practitioner will encounter, and it is invisible in a referral that describes a "disruptive class".

**The plan.** Four elements, and the plan is a plan for the whole class.

*Element one: change the antecedent.* The independent working period is broken into shorter segments with a defined check-in at the end of each, and Mr Adeyemi's circulation is planned, with a defined route that keeps him in view of the class, and with a defined task during the circulation — a "look at this" card that he uses to ask a student a question about their work, which gives him a legitimate reason to be at the back of the room and keeps his attention available to the whole class.

*Element two: the class contract.* The class agrees to a set of behaviours — three of them, drafted with the students — in return for a set of privileges that the class chooses, and the privileges are things the school can deliver: the last five minutes of the lesson for a chosen activity, a weekly slot in the school newsletter's quiz section, and a class vote on the music for the last lesson. The contract is displayed in the room, signed by the students, and reviewed at the half-term.

*Element three: the response to disruption.* The teacher response is specified in advance and is the change with the largest effect. When the noise rises, Mr Adeyemi does not address the class; he addresses the students who are working, and he says something that acknowledges them specifically — and the class is told in the first week that this is what happens. The three boys are not named, and the class is not told to notice who is being addressed. The logic is that the previous response was a public ritual of generalised disapproval, and the new response is a public signal that the working students are the ones the teacher's attention is for.

*Element four: the data.* Mr Adeyemi records the number of interventions and the number of students working at the end of each of the last ten minutes of the lesson, and the class's work output. The recording is minimal, and the review is with the practitioner every fortnight.

**The outcome.** The interventions drop substantially in the second fortnight and the working output rises. Mr Adeyemi's own summary is that the class is "not the same" and that the biggest change was not the class contract but the change in what he did when the noise started. The head of year withdraws the request to split the class.

**The interpretation.** The case illustrates the central theme of this chapter. The referral described three students; the analysis described a class-wide response pattern maintained by an adult's ritual, and the plan that changed the adult's behaviour changed the class. It also illustrates the theme of implementation: the plan required a teacher to change a deeply ingrained classroom habit, and the fortnightly review with the practitioner was the mechanism that sustained the change, because the old habit returns within days of the plan lapsing.

## 6. Further Reading for This Chapter

- Azrin, N. H., & Lanza, A. P. (1976). *How to use token economy*. — The classic practitioner text.
- Walker, H. T. (1992). *Classroom management*. — The most usable school text.
- Gagne, D. M., Yovanoff, P., & Butson, G. (2003). *Applied behavior analysis: Theory and practice for the school psychologist*. — The school-based applied text.
- Ward, H. M., Wolfe, J. L., Gutierrez, M. R., & Stowitschek, J. (2013). *A review of school-based behavioral interventions*. — The evidence review.
- Nelson, M. N., Smith, S. W., & Ogletree, M. R. (2015). *Behavior support for students with emotional and behavioral disorders*. — The contemporary school-based framework.
- Lane, H. A., Gac, S., & Grimes, K. (2016). *Supports-based behavioral interventions*. — The contemporary framework and its evidence.
- Kirschner, S. E., & Tompkins, M. A. (2014). *Using behavior change strategies in the schools*. — The practical handbook.
- Gresham, M. L. (1983). *Social learning and the reduction of classroom behavior problems*. — The foundational school-based study.
- Homer, J. B. (2015). *Reinforcement and contingency management with students*. — The applied guidance.
- Merenda, P. F. (2011). *Classwide function-related intervention teams: The classroom and the school*. — The classroom-level systems.
- Colton, A. (2009). *Classwide behavioral interventions: Filling in the missing link*. — The classroom-level evidence.
- Ministry of Education guidance on behaviour and discipline. — The jurisdictional framework; the practitioner must locate and read the local equivalent.
- Turnbull, H. R., et al. (2011). *Collaboration to improve outcomes for students with serious behavioral disabilities*. — The team-based legal framework.

# 7. Implementation: The Part That Determines Everything

## 7.1 The fidelity problem

The single most reliable finding in the school-based applied literature is that the outcome is determined by the fidelity of the implementation, and the school-based studies that measure fidelity and find it low report null results that reflect the implementation rather than the intervention. The practitioners who design programmes are systematically the ones who implement them least, because the work of teaching is what occupies the time.

Four measures make the difference, and they should be built into any plan from the beginning.

**A written protocol**, in the room, in short form, with the target behaviours, the responses, and the exception procedures.

**A staff training session**, including a rehearsal, and a refresher at the start of each term.

**A fidelity check**, in which the practitioner or a colleague records whether the response was delivered as specified. The fidelity check is uncomfortable and it is the most useful thing in the plan, because it converts "this isn't working" into "this is being delivered 40 per cent of the time".

**A short review cycle.** Fortnightly is usually right. A monthly review is too slow to catch a drift; a daily review is not sustainable for a practitioner with a caseload.

## 7.2 The problem of the adult who is not on the plan

Any school-based plan has to specify what happens with the adults who are not on it. The options are: train every adult who will have contact with the young person, which is the best option and is often impossible; or specify a small number of responses that any adult can apply, which is the second best; or accept the inconsistency and design the plan to survive it.

The third option is not a failure. A plan that survives inconsistent implementation is a robust plan, and robust plans are built on the antecedents and the consequences that are most widely available. The "first day" design of Chapter 25 is robust in this sense, because the young person's own action is the treatment.

## 7.3 The problem of turnover

Staff turnover destroys school-based interventions with a regularity that practitioners underestimate. A plan that depends on a named adult's consistent implementation is a plan with a shelf life of one or two terms.

Three responses help. Build the plan around the classroom environment rather than around the teacher's skill, so that it survives the teacher's departure. Train the whole class in the routine, so that the class maintains it. And put the plan in a form that a relief teacher can follow in thirty seconds.

## 7.4 The problem of the young person who is not in the room

Contingency management assumes the young person is present when the contingency operates. In a school, the young person is frequently not present: they are at the office, in the inclusion room, in the corridor, at the dentist, in a fixed-term exclusion. A plan that delivers a reward at the end of the lesson to a young person who was in the corridor for the last twenty minutes will teach the young person that the corridor is the place

to be, because the reward is available at the corridor's end too.

The specification should say explicitly what happens to the rewards accumulated during an absence, and in most systems the correct answer is that they are held and available on return, with a stated rule about the maximum accumulation.

### 7.5 The problem of the young person who declines

A young person who refuses to participate in a contingency programme presents a question that the practitioner should be ready for, and the answer depends on the young person.

Where the young person is refusing because the plan is not what they want, the plan should be renegotiated, because a plan the young person does not accept will not be implemented. Where the young person is refusing because they do not believe the system will be honoured, the first task is to honour it exactly as written for long enough to build the credibility, and the practitioner should say openly that this is what is happening. Where the young person is refusing because the behaviour is being maintained by something the plan does not address — and this is the most common reason — the plan is wrong and the functional assessment must be revisited.

## 8. Evidence and Research Support

### 8.1 The status of the evidence

The evidence for school-based behavioural intervention is substantial and mature, and it is strongest in three areas.

**Classroom-level interventions.** The class-wide function-related intervention team and its relatives have good support for reducing the level of disruption in a classroom, and the effect sizes reported in the meta-analyses are large, particularly on the proximal outcomes of on-task behaviour and teacher-directed talk. The limitations are the modest effect on the individual student's academic outcomes and the dependence on the teacher implementation.

**Token economies.** The evidence for classroom token economies is good for the reduction of disruptive behaviour, and the older institutional literature is extensive. The modern evidence is thinner than the practice.

**Function-based support plans.** Plans built on a functional behavioural assessment, and delivered consistently, produce better outcomes than plans built on the behavioural reduction model, and the finding is consistent with the applied evidence in Chapter 24.

**Contingency management for substance use.** The school-based and clinic-based applications of contingency management in the treatment of substance use have some of the largest measured effects in the psychological literature, and Chapter 47 develops this.

### 8.2 What the evidence says about implementation

Three findings are more useful to a practitioner than the outcome studies.

**Fidelity determines the outcome.** Where fidelity is measured, it is usually low, and the studies with high fidelity show larger effects.

**Training alone is insufficient.** Staff training improves knowledge and does not reliably improve implementation, and the addition of a written protocol, a rehearsal, and a monitoring process is what produces change in behaviour.

**Classroom-level systems have larger effects than individual-level systems** for classroom-wide outcomes, and the reason is that the classroom-level system changes the peer ecology, which the individual system cannot.

### 8.3 Limitations of the evidence

**The outcomes are proximal.** The studies measure the behaviour in the classroom, and the distal outcomes — the attainment, the family relationships, the mental health, the educational trajectory — are rarely measured, and the assumption that the proximal gains produce the distal gains is an assumption rather than a finding.

**The samples are selected.** The studies are frequently conducted with the classes and the students who are easiest to include, and the young people with the most severe needs and the least predictable attendance are systematically under-represented.

**The implementation conditions are exceptional.** The published studies are frequently conducted by the developers of the intervention, in schools with the resources to support the research, with the research team providing the training that a school would have to provide for itself.

**The evidence is Anglo-American and context-specific.** A token economy depends on the availability of certain backup reinforcers, on the currency of the tokens, and on the institutional culture of the school, and the transferability across systems is limited.

## 9. Strengths and Limitations of Contingency Management in Institutions

### 9.1 Strengths

**It is the clearest and most teachable form of the behavioural principles**, and it is the point at which the abstract concepts of reinforcement become visible and manipulable to a team.

**It produces rapid, measurable change**, which is essential in a setting where the young people who are referred have already experienced many slow interventions.

**It is compatible with the rest of the school's disciplinary framework**, because it specifies the consequences rather than replacing them.

**It involves the adults**, and the process of writing the plan with the teacher is itself a useful clinical intervention, because it forces the adult to state explicitly what they are currently doing and why.

**It can be adapted to any setting**, from a classroom to a clinic to a residential unit.

### 9.2 Limitations

**It is only as good as the implementation**, and school-based implementation is a chronically unreliable enterprise.

**It can be experienced as punishment.** A young person who is subject to a visible token economy in a school where the rest of the students are not may experience it as a punishment, and in some settings the removal of privileges in the name of a token system is indistinguishable from the removal of privileges in the name of a sanction.

**It is easily turned into a control system rather than a teaching system.** A system that removes more than it rewards, that is applied to a class rather than to a person, and that is not reviewed, has become a control system, and the literature on institutional practices makes this abundantly clear.

**It says nothing about why the behaviour occurs.** A young person whose disruption is maintained by a peer dynamic, by a learning difficulty, or by a family situation will not be helped by a well-run token economy, and the practitioner should be alert to the displacement of the problem.

**It is rarely faded.** A great many token economies in schools are in their third year, and the young people in them are correspondingly unable to work without them.

# 10. Cultural and Ethical Considerations

## 10.1 The market objection in the school

The ethical objection discussed in Chapter 26 is sharper in a school, because a school is an institution with an authority relation to the young people in it, and because the peers can see.

Four points follow, and they are the practical requirements of any school-based system.

**Never trade a right.** No system may condition access to education, to meals, to the toilet, to safety, to advocacy, or to a statutory process on behaviour. This is a legal requirement as well as an ethical one, and a practitioner who proposes a system that breaches it is proposing an unlawful arrangement.

**Never trade attention or warmth.** The adults' attention and warmth are not the currency. The specific practical prohibition is that the adults do not use praise and approval as the tokens, because that makes the adults' regard conditional on the young person's performance, and because it is the practice that most reliably produces a compliant and dependent young person.

**Never make the system public if it would humiliate.** For a young person who is already a target, the visible token chart is a hazard, and the design should protect them. The individual system should be delivered without a public display, and the class system should be designed so that the tokens are not individually visible.

**Do not let the system replace the relationship.** A class-wide system that reduces the disruption and fills the lesson with token administration can produce a classroom in which the teacher and the class no longer talk to each other, and this is a real and documented outcome, and the practitioner should watch for it.

## 10.2 Cultural considerations

**The reinforcers available in the school reflect the school's resources and values.** A school in a community with a strong culture of sporting achievement can offer access to the team; a school in a community where that is not valued cannot. The token economy must be built from what is genuinely available and genuinely valued in that community.

**The response to disruption is culturally patterned.** In some school cultures, a public correction is the ordinary way of maintaining order, and a practitioner who introduces a subtler response must be prepared to explain it and to support the teacher in it, because the teacher will be managing the reactions of colleagues.

**The status of the individual in the peer group determines the viability of the system.** A system that signals compliance as a virtue is received very differently by a young person whose peer group values defiance, and a practitioner should establish what the peer group rewards before designing a system that asks the young person to move against it.

**The discipline and the support frameworks are culturally and legally specific**, and a practitioner working outside the jurisdiction in which they trained must learn the local framework before writing a plan.

### 10.3 Ethical requirements

**Consultation and assent.** The young person and, where the system applies to a class, the class must be consulted. A system imposed on a group will be resisted, and the resistance is rational.

**Proportionality and the least restrictive option.** The consequences must be proportionate to the behaviour, and the less intrusive alternatives must be considered and recorded.

**Protection from the aggregation of power.** A behaviour support plan that concentrates the authority of several adults over one young person without review is a risk, and the plan should specify the review point at which the accumulation is examined.

**Documentation and the right to see it.** The plan, the data, and the reviews should be documented, and the young person and the family should be able to see them.

**Review with the family and with the young person, on a stated cycle**, with the question "is this plan still necessary?" included in the review.

**The end of the plan.** Every plan should have a stated end, and a plan with no end is a plan that has become the young person's accommodation.

**The recognition of the possibility of harm.** A practitioner who is introducing a system into a school should be able to state what harm the system might do, and how they would know, and what they would do. In most cases the answer involves the young person's peer relationships, and the answer is not usually "nothing".

## 11. Key Terms

**Key Terms at a Glance**

- **Contingency management in a setting** — The deliberate restructuring of the reinforcement structure of a classroom, a clinic, a ward, or a family, with specified behaviours, specified consequences, trained staff, and a review cycle.
- **Token economy** — A system in which specified behaviours are reinforced with tokens exchangeable for specified backup reinforcers.
- **Token** — The small, durable, immediately delivered item used as the conditioned reinforcer in a token economy.
- **Backup reinforcer** — The item or privilege for which the tokens are exchanged.
- **Exchange procedure** — The specified manner, time, and place at which tokens are converted into backup reinforcers.
- **Response cost** — The specified, small, and immediate loss of tokens following a targeted behaviour.
- **Fade plan** — The written schedule on which the token economy will be withdrawn, with the criteria for withdrawal.
- **Class-wide system** — A token economy applied to a whole class, with the consequences delivered to the class as a unit.
- **Individual system** — A token economy applied to a named young person for a specific target behaviour.
- **Contamination of the target behaviour** — The process by which the behaviour becomes contingent on the token rather than on the contingencies of the young person's life.
- **Behaviour support plan** — The written document specifying a young person's target behaviours, strategies, contingencies, adult roles, data collection, and review arrangements.
- **Graduated response procedure** — The specified, rehearsed, and consistent sequence of adult responses to increasing levels of disruption.
- **Escalation procedure** — The specified adult response to a young person's escalation, including the condition for the return to the setting.
- **Return condition** — The specified requirement that must be met before a young person removed from a setting returns, which prevents the escalation from being reinforced by the relief.
- **Fidelity of implementation** — The extent to which the specified adult responses were delivered as designed, measured independently of the outcome.
- **Written protocol** — The short, in-room document specifying the target behaviours, the responses, and the exception procedures.

**Key Terms at a Glance (continued)**

- **Staff rehearsal** — The practice, in a training session, of delivering the specified responses to a simulated disruption.
- **Refresher training** — The term-time repetition of the staff training, provided because the skills decay and the staff change.
- **Fidelity check** — The independent observation of whether the intervention was delivered as specified, which converts an unexplained null result into a diagnosable implementation problem.
- **Contingency inventory** — The assessment of what currently follows a behaviour in a setting, for the young person, the peers, and the adults.
- **Class contract** — A term-long written agreement between a class and the school, drafted with the students, in return for specified privileges.
- **Ritualised response** — A predictable adult response to a class-wide behaviour that functions as a signal of significance rather than as a consequence.
- **Function-related intervention team (FRIT)** — A classroom-level system in which a team of staff share the function of a behaviour and respond consistently, increasing the number of adults available to reinforce.
- **Manifestation determination** — The procedural requirement, under disability legislation, that a behaviour resulting in a change of placement be assessed against the possibility that it is a manifestation of a disability.
- **Functional behavioural assessment (FBA)** — The assessment required in many jurisdictions before a change of placement for a student with a disability, and the appropriate basis for a behaviour support plan.
- **Placement change** — A change in the educational setting of a student that triggers the procedural protections of the disability legislation in most jurisdictions.
- **Behavioural contract for a class** — An agreement between a class and the school in which the class commits to specified behaviours in return for specified privileges.
- **Special educational needs and disability code** — The jurisdictional framework that determines the legal status of a behaviour support plan and the procedures that must be followed.
- **Instrumental participation** — A young person's continued involvement in an intervention because it produces a benefit, without the corresponding development; the risk of an unfaded system.

## 12. Chapter Summary

Contingency management in schools and clinics is the most direct application of the operant principles of Chapter 26 to real settings, and its central difficulty is not technical. A school is an organisation of many adults, each of whom responds to a young person's behaviour, and a contingency system is only as strong as its weakest response. The failure mode is predictable: a programme is designed with the class teacher, the teaching assistant is not told, the supply teacher is not told, and on the first day the young person learns that

the system holds on Tuesday and not on Wednesday.

The chapter's first idea is that every setting is already a contingency system, and that the question is never whether to introduce consequences but whether the existing ones serve the young person. The contingency inventory is the assessment tool, and its three common findings are that the problem behaviour is reinforced, that the desired behaviour is unreinforced, and that the adults are inconsistent.

The conditions for appropriateness were stated and the conditions for inappropriateness were stated with them, and the special caution about young people with severe cognitive impairment — for whom the tokens must be understood and the fade correspondingly slower — was included. The token economy was described in its six design decisions and its four implementation requirements, and the class-wide and individual systems were contrasted, with the hybrid recommended. The problems specific to token economies were set out: the contamination of the target behaviour, the peer problem, the staff problem, the incompatibility of token delivery with exclusionary responses, and the risk of promising a reward the school cannot deliver.

Behaviour support plans were described in their elements, and the escalation procedure was given the attention it deserves, because an adult's improvised response to a young person's escalation is the treatment, and because the most common errors — removal without a return condition, variable duration, and return on calming — all inadvertently reinforce the escalation. The chapter noted the legal framework that gives the plan a standing beyond the clinical one, and the practitioner is directed to establish the local regulatory position before writing a plan.

The case of Mr Adeyemi's class illustrated the whole chapter. The referral described three students; the analysis of the data described a single Tuesday afternoon lesson and a thirty-minute unsupervised activity; the functional analysis described a ritualised adult response that functioned as a signal of significance, and the plan that changed the adult's behaviour changed the class. The class contract was, in the end, the smallest part of the intervention.

The implementation section addressed the fidelity problem, the adult who is not on the plan, staff turnover, the young person who is not in the room, and the young person who declines, and the section on evidence noted that the school-based literature is mature and proximal, that fidelity determines the outcome, that training alone is insufficient, and that the classroom-level systems have larger effects than the individual ones because they change the peer ecology.

The ethical analysis in a school setting is sharper than elsewhere because a school is an institution and the peers can see, and the four prohibitions were stated without qualification: never trade a right, never trade attention or warmth, never make the system public if it would humiliate, and do not let the system replace the relationship. The cultural analysis addressed the community-specificity of the available reinforcers, the cultural patterning of public correction, the peer group's valuing of defiance, and the jurisdictional specificity of the discipline framework. The ethical requirements closed the chapter with the question that every review should contain: is this plan still necessary?

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the thesis that every setting is already a contingency system.
2. Describe the contingency inventory and state its three common findings.
3. State the three features that make schools and clinics appropriate sites for contingency management.
4. State the six conditions under which contingency management is appropriate, and the four circumstances in which it is not.
5. Explain the special caution required for young people with severe cognitive impairment.
6. State the six design decisions required for a classroom token economy.
7. Describe the five implementation requirements for a token economy, and identify the one most often omitted.
8. Compare the class-wide and the individual token systems, and explain the basis of the recommendation for a hybrid.
9. Explain the contamination of the target behaviour and the role of the fade plan in it.
10. Explain the peer problem, the staff problem, the dismissal problem, and the reward problem in token economies.
11. State the eleven elements of a behaviour support plan.
12. Describe the graduated response procedure and the escalation procedure, and state the three most common errors in them.
13. Explain the significance of the return condition, and describe the effects of the three common errors.
14. Explain the legal and regulatory context of a behaviour support plan, and name the framework in the United States and in the United Kingdom.
15. Describe the form and the advantages of the class contract, and state its three disadvantages.
16. State the limits that apply to a behaviour contract in a school, and explain why each follows from the law.
17. Describe the case of Mr Adeyemi's class and state the two features of it that generalise.
18. Explain the four implementation problems: fidelity, the adult not on the plan, turnover, and the young person not in the room.
19. Explain the three possible reasons for a young person's refusal to participate, and the appropriate response to each.
20. Summarise the status of the evidence for classroom-level interventions, token economies, function-based support plans, and contingency management for substance use.

**Review and Discussion Questions (continued)**

21. State the three implementation findings from the evidence, and explain why they are more useful to a practitioner than the outcome studies.
22. State the four limitations of the school-based evidence base.
23. State the five strengths and the five limitations of contingency management in institutions.
24. State the four prohibitions that apply to a school-based system, and explain the reasoning for each.
25. State the three cultural considerations that bear on the design of a school-based system.
26. State the eight ethical requirements for contingency management in a school, and explain the significance of the requirement that every plan have an end.

## 14. Practice Activities

**Practice and Application**

### Activity 28.1 — The Contingency Inventory (90 minutes, field-based)

In a placement, choose one behaviour that concerns the adults in the setting but that the referral describes in general terms. Complete the contingency inventory for it: what happens immediately before, what happens immediately after, for the young person, the peers, and the adults, and what the young person's alternative produces. Write the inventory in one page. Then write the single sentence stating what the behaviour appears to accomplish. Most students find that the behaviour is being reinforced by something nobody intended, and the finding is almost always more actionable than the behaviour.

### Activity 28.2 — Designing a Token Economy (90 minutes, individual)

Design a classroom token economy for a real secondary class. Specify the three target behaviours, the token, the backup reinforcers, the schedule, the exchange procedure, the response cost, and the fade plan. Then write the protocol as it would appear on the desk, in no more than twenty lines. The twenty-line constraint is the discipline, and most first drafts are twice as long as they should be and cannot be used at the front of a room.

### Activity 28.3 — The Fidelity Simulation (60 minutes, small group)

In groups of four, one student plays the teaching assistant, one the subject teacher, one the young person, and one the practitioner observing. The practitioner describes a plan and the two staff members respond to a simulated disruption in the way they think appropriate, without further instruction. The practitioner then checks the response against the written protocol. The exercise is uncomfortable and it is the most accurate simulation of the implementation problem that the book contains, and it consistently shows that the discrepancies are not in the staff's motivation but in their knowledge of what the plan requires.

### Activity 28.4 — Redressing a Plan (60 minutes, individual)

Take a behaviour support plan from a placement, or construct one, and audit it against five questions: does it condition any right on behaviour; does it use adult attention or approval as a reward; is it visible to peers in a way that would humiliate a particular young person; does it specify what happens with the adults who are not on the plan; and does it have a stated end? For each failure, write the revision. Most plans fail the first three, and the revisions are the practical work of the chapter.

## 15. Annotated Reading Pathways

**For the design.** Read Azrin and Lanza (1976) for the token economy, and read the classroom-level systems literature for the function-related intervention team and its relatives. The classroom-level material is the most direct guide to the class-wide approach and it has the strongest effect sizes.

**For the implementation.** Read the fidelity literature, and read one study that measured implementation and reported a null result. The contrast with a study that measured implementation and reported a large effect is the most useful pair of readings in the chapter.

**For the law.** Read the local behaviour and disability framework before writing any plan, and read the manifestation determination guidance. This is not optional reading and it takes an afternoon.

**For the evidence.** Read the school-based reviews and the classroom-level meta-analyses, and read the substance use contingency management literature, which is the most methodologically mature part of the applied evidence base.

**For the ethics.** Read the critical literature on reward-based practice in schools and on the historical use of token economies in institutions, and read the disability rights literature on the protections of students with behaviour difficulties. The combination of the technical and the critical literature is the preparation for writing a plan that a school will use and a family will accept.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Contingency Management in Schools and Clinics** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is token systems, behaviour contracts, and the negotiated restructuring of reinforcement in real settings. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the

next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tunde is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tunde's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tunde explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Tunde chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tunde has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tunde can describe the pattern without

adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?

- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Contingency Management in Schools and Clinics

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Contingency Management in Schools and Clinics** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Contingency Management in Schools and Clinics** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

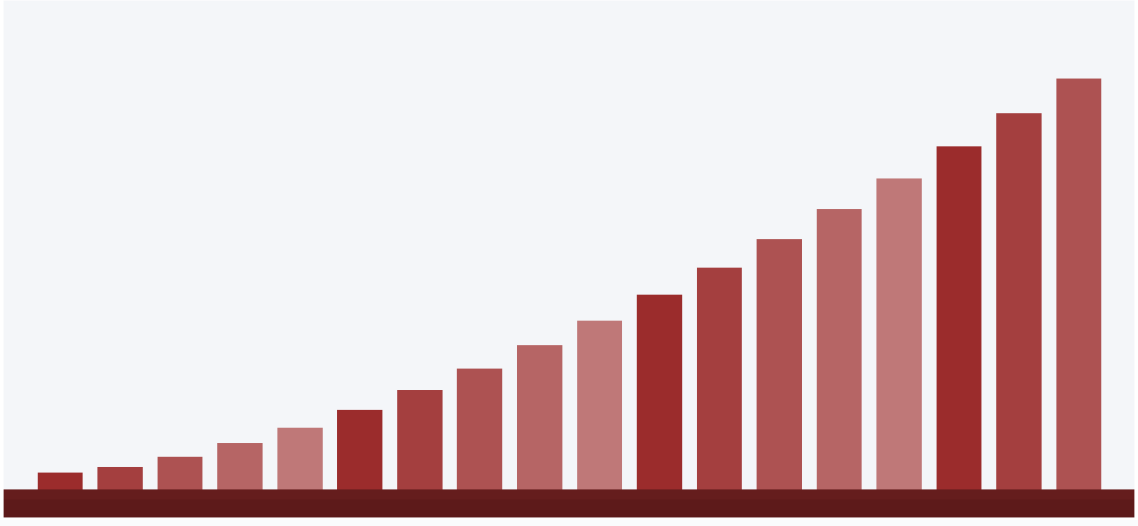
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Contingency Management in Schools and Clinics**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Contingency Management in Schools and Clinics to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 29

# Functional Analysis of Self-Injurious and Aggressive Behaviour

Safety-oriented assessment and intervention with behaviour that is harmful or dangerous.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the safety-first framework that governs work with self-injurious and aggressive behaviour.
- Describe the epidemiology, the range of presentation, and the differential diagnosis of self-injury and aggression in children and adolescents.
- Describe the functional analysis of self-injurious behaviour, including the assessment of the antecedents, the behaviours, and the medical factors.
- Apply the four functional classes and the setting demands to the analysis of self-injury and aggression.
- Describe the safety interventions for self-injury and aggression, including the physical, environmental, and relational components.
- Describe the treatment approaches for each function, and state the specific treatment of aggression maintained by escape and by attention.
- Explain the standards of care and the documentation required for work with this behaviour.
- Critically evaluate the cultural and ethical dimensions of work with dangerous behaviour, including the questions of restraint, of restrictive intervention, and of the young person's dignity.

## Chapter Overview

This chapter concerns behaviour that can kill or permanently injure a young person, and it is written with a different set of conventions from the rest of the book. There is no discussion here of whether the behaviour should change. The behaviour must change. The clinical questions are about what the behaviour is accomplishing, what the safest intervention is, and what treatment will be effective and safe in the long run.

Two behavioural presentations are treated together because they frequently co-occur, because the analysis is parallel, and because the young people who present with one frequently present with the other later. They are, however, distinct in their consequences, their risk profiles, and their treatment implications, and the chapter keeps them distinct.

The safety framework has four elements and it should be stated at the outset because it governs everything else in the chapter. **The safety of the young person comes first and is not contingent on the assessment being complete. The least restrictive effective procedure governs the choice of intervention. The treatment of the behaviour is not the treatment of the condition**, and a young person whose self-injury is a symptom of a disorder requires the treatment of the disorder, not only the management of the injury. And **the intervention must be accompanied by a plan for the young person's dignity**, because the interventions that protect a young person from harm are frequently experienced as punishments, and the young person who experiences them as punishments will not tell anyone when the next episode is building.

A working knowledge of this material is not optional for a practitioner who works with young people, and a working knowledge of the ethical framework is not optional either, because the interventions in this area are the ones in which a practitioner's decisions can be challenged and reviewed.

## 1. Historical Context

## 1.1 From the asylum to the institution

Self-injurious behaviour has a long and grim history in the institutional treatment of people with learning disabilities and mental illness. The nineteenth-century practice of "moral treatment" included elaborate mechanical devices designed to prevent self-injury, and the twentieth century added the padded cell, the restraint, and the surgical intervention for the self-injurious behaviour of a small number of people with a severe genetic condition. The literature is a record of a systematic failure to understand the behaviour before trying to control it, and it is a useful reminder that an unexplained behaviour is usually treated harshly.

## 1.2 The functional analysis of self-injury

The decisive development was the work of Iwata and colleagues from the 1980s, and its central finding was methodological. Before that work, self-injury was treated on the assumption that it served an unknown purpose, and the intervention was a physical one. Iwata's group demonstrated that the behaviour could be analysed experimentally in the same way as any other behaviour: by manipulating the consequences across conditions and observing whether the behaviour changed.

The findings were published in 1982 and 1994 and they are the foundation of everything that followed. Self-injury in the population studied was overwhelmingly maintained by **automatic reinforcement** — the behaviour produced its own sensory consequence — and, in the remainder, by escape from demands and by attention. The proportion of cases in which the behaviour was maintained by automatic reinforcement was much higher than had been assumed, and this finding changed practice fundamentally, because it meant that a young person whose self-injury is automatically maintained requires a very different plan from one whose self-injury is escape-maintained.

The therapeutic consequence of the automatic reinforcement finding is the treatment of the sensory consequences themselves, through the pharmacological management of the underlying condition, through a programme of alternative sensory stimulation, and through the extinction of the behaviour — and the practical consequence is that the "treatment as usual" of self-injury, which was containment, is largely ineffective, because containment removes the behaviour but does not remove the maintaining contingency.

## 1.3 The function of aggression

The functional analysis of aggression developed in parallel, with the critical contribution of the studies of aggression maintained by **attention** and by **escape**, and the demonstration that a substantial proportion of physical aggression in young people with learning disabilities is escape-maintained, is a request for help, or is the only available form of communication, and not a desire to hurt.

This finding has had a profound effect on practice, because the response to a young person's aggression in most settings is escalation, and escalation is exactly what maintains the behaviour.

## 1.4 The regulatory history

Work with this behaviour in the United States is now subject to a dense regulatory framework, and a practitioner must know it before working in a setting where it applies. The relevant framework includes the federal protections for individuals with disabilities, including the requirement of a functional behavioural assessment before a change of placement and the manifestation determination procedure; the prohibitions on the use of seclusion and of physical restraint as written in the federal regulations, and the conditions under which they may be used; the requirements of the Americans with Disabilities Act in relation to the

modification of policies and practices; and the state-level licensing standards for residential and inpatient settings, which vary considerably and are frequently more specific than the federal rules.

The practical requirement is straightforward and should be stated in the chapter's conclusions: a practitioner working with this behaviour in a regulated setting is implementing the plan of a team, under supervision, within a documented protocol, and every element of the plan that touches the young person's body, liberty, or dignity is subject to review.

# 2. Safety

## 2.1 The hierarchy

The safety hierarchy for work with dangerous behaviour is ordered, and the order is not negotiable.

**Level 1: Immediate physical safety.** The prevention of imminent harm to the young person or to others. This is the only consideration that overrides every other consideration, including the young person's assent, the family's preference, and the practitioner's own discomfort.

**Level 2: Medical assessment.** The determination that the injury or the aggression is not a medical emergency requiring other care. A young person who is self-injuring may have an undiagnosed condition contributing to it, and a young person who is aggressive may be in pain, or have a neurological condition, or be in a state of delirium, and in each of these cases the behavioural analysis is the wrong frame entirely.

**Level 3: The functional assessment.** The determination of what the behaviour is accomplishing. This cannot be conducted while the young person is in acute danger, and a plan that requires the assessment to be completed before the safety interventions are in place is a plan that will fail.

**Level 4: The treatment.** The long-term reduction of the behaviour and the increase of the young person's repertoire.

## 2.2 The safety plan for self-injury

A safety plan for self-injury has five components, and it is written down, shared with everyone who works with the young person, and rehearsed.

**The functional prediction.** The circumstances in which the behaviour is likely, stated specifically enough to be recognisable, and derived from the functional analysis if it has been conducted, and from the pattern of previous episodes if it has not.

**The response to the early indicators.** The physiological and behavioural precursors — a change in breathing, a change in the pattern of movement, the sudden quietness, the change of tone — and what the adult should do at that point. The early indicators are specific to the individual and are usually known only from careful observation, and they are the most valuable single piece of information in a safety plan.

**The response to the behaviour.** The procedure, specified in advance, and practised, and consistent across all adults. The procedure should specify the physical approach, the words used, the position of the adult, and what is done if the young person resists.

**The provision of the alternative.** The sensory input or the break or the help that the analysis indicates the young person is seeking, available at the moment of the early indicators and not only after the behaviour.

**The medical and the psychological response after an episode.** The physical care, the documentation, the review, and the decision about whether the episode changes the plan.

## 2.3 The safety plan for aggression

The safety plan for aggression has five components as well, and the first of them is different in kind.

**The identification of the target of the aggression.** Who is at risk: the young person, other young people, staff, family, or the wider community. The risk assessment should be explicit about the worst credible outcome, and the practitioner should be willing to state that a young person has a realistic potential to kill, because the risk assessment is useless if it understates the risk.

**The identification of the precursors**, as for self-injury, with the addition of the situational variables: the time of day, the staffing level, the noise level, the presence of a specific peer, the state of the young person's medication.

**The avoidance and the environmental modification.** The changes to the environment that reduce the likelihood of the episode — the change of route, the reduction of noise, the staffing adjustment, the removal of the trigger from the space.

**The response to the behaviour**, specified and rehearsed, and including the specification of what the adult does when the young person is escalating, which is usually the most important part of the plan and the part most often improvised.

**The plan for the aftermath**, including the repair of any relationship damaged by the episode, which is a clinical task and not a courtesy. A young person who has just attacked a member of staff and is then treated as a danger for a month has learned that violence is the only route to significance, and the plan for the aftermath is the part of the plan that prevents the next escalation.

## 2.4 The role of restrictive interventions

Restrictive interventions — physical restraint, seclusion, and the chemical equivalents — are the most fraught part of this work, and the professional position should be stated plainly.

They are **last-resort interventions**, used only when the young person poses an imminent danger to themselves or to others, and when less restrictive interventions have been tried and have failed, and when the intervention is used for the shortest possible time, and with the monitoring and the documentation and the review that the law requires.

They are **more likely to cause harm than to prevent it** in the cases in which they are used most often, because they are used with the young people who are most distressed and least responsive, and the restraint of a distressed young person is itself a trigger, and it frequently produces the aggression it is intended to prevent.

They are **subject to oversight**, and a practitioner who uses them without a documented protocol, without the training, and without the review is not practising within the professional standard, whatever the local law happens to permit.

And they are, in the clinical literature, associated with higher rates of injury to the staff, higher rates of injury to the young person, higher rates of subsequent aggression, and worse therapeutic outcomes. This is not an argument against their use in a genuine emergency; it is an argument for the seriousness with which they should be used, and for the institutional responsibility to design the environment so that they are

needed less.

# 3. Presentation and Diagnosis

## 3.1 Self-injurious behaviour

Self-injury in children and adolescents presents in a wide range of forms and with a wide range of functions. The forms include: skin picking, scratching, and cutting; head banging and hitting; eye poking; hair pulling; and the more severe forms involving burning or the ingestion of objects. The frequency ranges from occasional and low harm to several times daily and severely injurious.

The differential diagnosis is wide and includes: the stereotyped movement disorders of the autism spectrum; the self-injury associated with specific genetic syndromes, notably Lesch–Nyhan syndrome, Smith–Magenis syndrome, and Cornelia de Lange syndrome; the self-injury associated with a psychotic disorder; the self-injury associated with a trauma history, including the self-harm behaviours of a young person who is being abused; the self-injury associated with a mood disorder; and the self-injury associated with a communication impairment or with an unmet need.

The differential has direct clinical consequences, because the treatment of the self-injury follows the diagnosis. Self-injury in a young person with a genetic syndrome with a characteristic neurochemical profile may be substantially responsive to pharmacological management, and the behavioural plan is then an adjunct rather than the treatment. Self-injury that is a symptom of psychosis requires the treatment of the psychosis. Self-injury that is a response to abuse requires the protection of the young person, not the extinction of the symptom.

## 3.2 Aggression

Aggression likewise presents in a range of forms: verbal aggression and threats; the destruction of property; physical aggression towards peers, towards adults, and towards animals; and the more severe forms of severe violence.

The differential diagnosis includes: the impulsivity and irritability of a mood disorder, including the irritability of a depressive episode, which is a presentation that is frequently missed in adolescents and which is often the correct diagnosis of a young person referred for aggression; the impulsivity of a neurodevelopmental condition; the psychosis, in which aggression may be a response to persecutory beliefs; the disinhibition of a substance; the dysregulation of a trauma response; the learned pattern of a young person who has been in an environment where aggression was the only effective response; and the communication of a young person who has no other way to express a need.

The diagnosis of the mood disorder is the one most often missed and it is worth dwelling on. An adolescent with a depressive episode, with the irritability that is its characteristic presentation in adolescence, is frequently referred to a behaviour service because the irritability presents as aggression. Treating the aggression behaviourally, without treating the depression, will fail, and it will fail in a way that increases the young person's shame.

## 3.3 The assessment

The assessment of dangerous behaviour is comprehensive, and its components are as follows.

**A full history**, including the developmental history, the medical history, the medication history, the trauma history, the family history, the educational history, and the history of previous interventions and their outcomes.

**A physical examination and a medical review**, including a review of the medication, a review of the pain, a review of the sleep, a review of the seizures, and a review of the sensory environment. A young person in pain is an aggressive young person, and this is not a figure of speech.

**The functional analysis**, which is the subject of Sections 4 and 5.

**A risk assessment**, which is a separate document from the functional analysis and has a separate structure: the identification of the risk, the likelihood, the severity, the protective factors, the current level of risk, and the review date. The risk assessment should be written in language that a court, a family, and a young person could each read and understand.

**A functional assessment of the environment**, which examines the antecedents in the physical setting: the noise, the crowding, the lighting, the temperature, the seating, the transitions, the staff-to-young-person ratios, the schedule.

**A developmental assessment**, because the meaning of the behaviour and the feasible alternatives are both developmental facts.

**An assessment of the young person's own view.** A young person who is aggressive has usually been asked about it many times and has usually found the question useless, and the practitioner should approach it differently, with attention to what the young person wants, what makes the behaviour difficult to stop, and what the young person has tried.

# 4. The Functional Analysis of Self-Injury

## 4.1 The stages

The functional analysis of self-injury proceeds in a defined order, and the order reflects the safety hierarchy.

**Stage 1: Safety and medical.** The safety plan is in place, the medical assessment is complete, and the physical management of the injury is available.

**Stage 2: Descriptive assessment.** A structured record of the episodes, their antecedents, their consequences, the settings, the times, the frequency, the severity, and the injury sustained. The descriptive assessment is the foundation of the experimental analysis, and it is also the point at which the pattern of the early indicators is discovered, which is the most clinically valuable output of the whole exercise.

**Stage 3: Experimental analysis.** The manipulation of the consequences across conditions.

**Stage 4: The treatment plan**, specified in terms of the function.

## 4.2 The experimental analysis

The analysis of self-injury uses a set of conditions, each of which is designed to isolate a particular consequence. The conditions run in blocks, typically five to fifteen minutes, in a treatment room or a ward, with the young person observed and the behaviour recorded.

**The alone condition** — the young person is alone in the room, with an observer. The behaviour that persists in this condition is maintained by its own consequences, that is, automatically.

**The attention condition** — the observer attends to the young person, and the attention is delivered in the way that the descriptive assessment suggests the young person finds reinforcing, which is usually the way in which the behaviour has previously obtained attention. The behaviour that increases in this condition is maintained by attention.

**The demand condition** — demands are presented, and the opportunity to escape or to take a break is contingent on the behaviour. The behaviour that increases under demands is escape-maintained.

**The play condition** — the environment is arranged to be engaging and the observer does not attend to the self-injury. The behaviour that decreases in this condition is, by elimination, automatically maintained.

**The control conditions** — the condition in which the consequence of interest is delivered non-contingently, which is the control for the possibility that the mere presence of the consequence is maintaining the behaviour.

**The social control condition** — the young person is observed with another person present who does not respond to the behaviour, which distinguishes the behaviour that requires an audience from the behaviour that does not.

The interpretation follows the logic of Chapter 24. The behaviour that is high in the alone and play conditions and low in the attention and demand conditions is automatically maintained. The behaviour that is high in the attention condition is attention-maintained. The behaviour that is high in the demand condition is escape-maintained. A great many cases show more than one pattern, and the analysis then describes a behaviour with more than one function, which requires a treatment plan for each function.

### 4.3 The interpretation

The interpretation of the analysis has three parts: the function, the setting demands, and the establishing and maintaining conditions.

**The function** is what the behaviour produces: sensory input, escape from a demand, escape from social interaction, or attention from others.

**The setting demands** are the conditions under which the behaviour occurs: automatic, attention, escape, or the combinations.

**The establishing conditions** are how the behaviour came to be performed, and they are usually historical — a period of illness in which the behaviour produced an analgesic effect, a period of abuse in which it produced a change in the perpetrator's behaviour, a period of communication impairment in which it was the only effective signal.

The establishing conditions matter because they are the reason the behaviour is "stuck", and because they indicate that the treatment must address something other than the current contingencies alone.

## 5. The Functional Analysis of Aggression

### 5.1 The procedure

The analysis of aggression follows the same structure, with three modifications.

**The analysis is conducted in the settings in which the aggression occurs**, whenever this is possible, because the contingencies in a ward and the contingencies in a classroom are different, and the behaviour is

often maintained by different consequences in each.

**The observation is of the antecedents with particular care**, because the predictors of serious aggression in a setting are often specific and physical: the noise, the crowding, the time of day, the specific peer, the transition, the medication state, the pain.

**The consequences are examined with attention to the adult responses**, because the adult response to the escalation is usually the maintaining contingency. The analysis includes a record of what the adult did immediately after the aggression, and the analysis of the escalation sequence is usually the most informative single component.

## 5.2 The four patterns and their treatments

**Attention-maintained aggression.** The aggression reliably produces adult attention, and the treatment is differential reinforcement of the appropriate behaviour and the planned ignoring of the aggression, and — this is the difficult part — the systematic change of the adults' response so that the escalation no longer produces attention. The treatment also requires the adults to provide the attention at other times, so that the young person is not deprived of it altogether.

**Escape-maintained aggression.** The aggression reliably terminates a demand. The treatment is differential reinforcement of compliance, the planned ignoring of the escalation, the reduction of the demands, and the teaching of an alternative request for help or break. The treatment must include a clear plan for what the adults do when the escalation occurs, because the escalation is the behaviour that is working.

**Automatically maintained aggression.** The aggression produces its own sensory consequence, which is uncommon in aggression as distinct from self-injury, and it occurs most often in young people with a neurodevelopmental condition and a sensory processing difference. The treatment is the management of the sensory environment and the pharmacological treatment of the underlying condition.

**Multiple functions.** A great many young people present with aggression maintained by both escape and attention, and the treatment must address both, and the differential reinforcement of compliance with the extinction of escalation is the core of the plan in either case.

## 5.3 The treatment of the escape-maintained case in detail

Because the escape-maintained case is both the most common and the most reliably treated, it is worth setting out the procedure in the detail in which it should be implemented.

**Reduce the demands.** The demands that the young person is escaping are identified and broken down into smaller steps, and the young person is taught the steps. A young person who cannot do the task will escape the task, and the correct intervention is the teaching of the task.

**Reinforce compliance.** Every instance of compliance is reinforced, immediately, with something small and frequent. The reinforcement must be more immediate and more frequent than the escape it is intended to replace, and the schedule must be dense at the beginning.

**Do not respond to the escalation.** The escalation is ignored — genuinely ignored, with no eye contact, no speech, no raised voice, and no attempt to calm the young person. This is difficult for adults, and it is the part of the plan most likely to fail, and it is the part most in need of training and rehearsal.

**Provide the alternative.** A signal for help, a break card, a specified call to a staff member, which the young person can use, which is trained, and which is honoured immediately when used. The alternative must be

available at the same time as the demand, and it must be a real alternative and not a slower route to the same escape, because a young person who can ask for a break in thirty seconds will use it.

**Expect the burst.** The behaviour will increase before it decreases, and the practitioner should tell the young person this in advance, and should not interpret the increase as a failure of the plan.

**Fidelity.** The adults' responses must be consistent, and the fidelity of the response to the escalation is the variable that determines the outcome.

## 6. Further Reading for This Chapter

- Iwata, J., et al. (1982). Toward a functional analysis of self-injury. *Journal of Consulting and Clinical Psychology, 50*(4), 600–606. — The foundational paper.
- Iwata, J., et al. (1994). *The assessment and treatment of self-injury*. — The full procedure.
- Aman, J., et al. (2000). *The treatment of self-injury and related behaviors*. — The research and clinical handbook.
- Rochester, M., & Mather, R. (2013). *The assessment and treatment of self-injury and aggression in developmental disabilities*. — The comprehensive clinical text.
- Matson, J. L., & Matson, M. L. (2015). *Assessing and treating aggression in developmental disabilities*. — The modern clinical approach.
- Sparrow, W. D., McIntire, D., & Shelty, V. (2011). *Autism spectrum disorders: From theory to practice*. — The assessment and medical framework.
- Symes, B. (2009). *Self-injury and the treatment of people with intellectual disabilities*. — The clinical and ethical framework.
- McDonough, M. A., et al. (2014). *Behavioral treatments for self-injury in children with intellectual disability*. — The treatment evidence.
- Slifer, A. R., et al. (2011). *A review of treatment interventions for self-injurious behavior in individuals with intellectual disabilities*. — The treatment review.
- Doser, N. M., et al. (2013). *The assessment and treatment of aggression in autism spectrum disorders*. — The population-specific approach.
- McGillis, T., et al. (2021). *Assessment and treatment of self-injury and aggression in developmental disabilities*. — The current practice.
- U.S. Department of Education regulations on the use of seclusion and physical restraint. — The regulatory framework; the practitioner must locate and read the local equivalent.
- Association for Behavior Analysis International (2014). *Professional practice and ethics*. — The ethical requirements.
- National Association of School Psychologists (2020). *Position statement on the use of seclusion and restraint*. — The professional position.
- Weatherby, N. A., & Torrey, E. C. (2012). *Functional analysis of problem behavior: A complete evaluation guide*. — The practical procedure.

## 7. Case Illustration: The Young Person Who Hurt Himself in the Bathroom

**Presentation.** Marcus is fifteen, and he lives in a residential placement. He has a diagnosis of a neurodevelopmental condition and a history of placement breakdown. He has a behaviour of biting his own forearm, most often in the bathroom, most often in the late afternoon, and it has been increasing over the last six weeks. The injury on the day of the referral was severe enough to need a hospital assessment, and the placement manager is asking whether the placement can continue.

This is the situation in which a practitioner must be extremely careful about the sequence of events, because the pressure is to produce a plan immediately and the plan that gets produced under this pressure is usually a containment plan that will not work.

**Step 1: safety.** The safety plan is written on the first day, before anything else, and it is short and it is specific. It names the early indicators — and these come from the staff, who have been the ones watching: a particular stillness, the sudden interest in the bathroom, the way he stops mid-sentence, a hand going to his pocket. It specifies who goes to him, what they say, and what they do. It specifies the response to the behaviour itself, which is the removal of the identified object — in this case, it is a modification of his clothing, because the biting has been found to occur when his forearm is bare and the arm is exposed, and the clothing change is a modification that costs him nothing in dignity. And it specifies the provision of the alternative: a weighted item that he takes with him and that he has already identified as calming, which is permitted and encouraged, and which is the thing that has been removed from him in the last placement.

**Step 2: medical.** The medical review is the second step, and it is not a formality. It establishes a full medication review, and it establishes that Marcus's last medication review was eleven months ago, which is itself a finding. It establishes that he has been having significant headaches, which his presentation team had not known, and which are being treated. It establishes that his sleep has been disrupted by the placement's night-time routine, and that he is not eating properly, and that both have changed in the last six weeks, which is the period over which the behaviour has increased. A change in the behaviour over a defined period always indicates a change in the conditions, and the medical review is where the change is found.

**Step 3: the functional assessment.** The descriptive assessment comes next, and it is done with the staff rather than in a clinic, because the behaviour occurs in the placement.

The ABC record is revealing. The antecedents are consistent: late afternoon, the bathroom, alone, and — in the last six weeks — the arrival of a specific member of staff who works the late shift and who has come from a different placement with a very different approach, and who is loud, and who rearranges the bathroom. The consequences are also consistent: the bathroom door is closed, and the young person is alone, and the physical sensation of the bite is a strong stimulus, and the bite is followed by a reduction in the emotional state that the staff describe as "settled" and that Marcus describes, much later, as "quieter".

The pattern is that the behaviour is most likely in the late afternoon, that it has increased since a change of staff, and that it is followed by a marked reduction in the young person's distress. That pattern is consistent with automatic reinforcement — the behaviour produces a sensory and affective consequence — and it is also consistent with escape, because being alone in the bathroom is a relief after a shift with a loud staff member.

**Step 4: the experimental analysis.** The analysis is conducted in a structured way in the placement's quiet room. The conditions are: alone; play; attention; demand; and the corresponding controls. The results are

clear and they are not the results the placement team expected.

The behaviour occurs at a high rate in the alone condition, at a low rate in the play condition, at a low rate in the attention condition, and at a low rate in the demand condition. The behaviour is **automatically maintained**, and the placement team's expectation — that the behaviour was a bid for attention, which was the reason their previous placements had escalated to restraint — was wrong.

**The establishing conditions.** The behaviour is not new; it existed, more mildly, in the previous placement, and it was described there as attention-seeking. The question of what it was originally for is not answerable from the records, and the honest answer is recorded.

**The treatment.** The plan has four elements.

*The medical and environmental work first.* The headaches are treated. The sleep and the eating are addressed, because a young person in pain and in a state of sleep deprivation is more likely to self-injure, and because these were the changes that preceded the increase. The loud staff member is consulted — with a discussion about the approach rather than about the young person, and with the specific request that the bathroom not be rearranged during the late shift, which is a small change with a large effect.

*The modification of the environment.* The weighted item is returned, and its use is not treated as a problem. The bathroom door is not closed on him; the door has a gap and he is visible. The clothing modification is made permanent for the late afternoon, and the young person chooses the clothing, and the choice is the point.

*The differential reinforcement of the alternative.* The alternative behaviour — going to the quiet room with the weighted item when the distress builds — is reinforced immediately, and the reinforcement is frequent and small, and the adult response to the alternative is warm and specific.

*The extinction of the behaviour, with a safety overlay.* The behaviour is placed on extinction: the consequence that has been maintaining it, which is the quiet and the relief, is no longer delivered immediately, and the bathroom is no longer available as a place of immediate relief, because the consequence of the behaviour is now the intervention of the staff, which is the safety response. This is the step that is most likely to produce an increase in the first week, and the staff are told so in advance, and the safety response is specified in the plan so that the increase does not become a crisis.

**The outcome.** The behaviour decreases over approximately six weeks, with a consistent pattern of a slight increase at the point of each staff change and a rapid decrease once the new staff member is established. Marcus's placement continues. The medication review is now monthly. The sleep and the eating have improved.

**The outcome that the plan was not designed to produce.** Marcus's own account, offered in a session in the eighth week, is that the biting was "the only way to make it stop" — and the "it" is the noise and the rushing and the being told what to do. The behavioural analysis found the automatic consequence and the treatment addressed it, and the young person's own account found the escape and named it precisely. Both were correct, and the plan that had been built only on the experimental analysis would have addressed the automatic function and left the escape function intact, and the behaviour would have persisted in a modified form.

The case is therefore an argument for the method described in Chapter 24: the young person's own account is data, and it should be gathered as early as possible, and it is not an alternative to the experimental analysis but a supplement to it.

# 8. Evidence and Research Support

## 8.1 The status of the evidence

The evidence for the treatment of self-injury and aggression is more methodologically mature than the evidence for most of this book, and this is because the behaviour is severe enough, and the outcomes are severe enough, and the field has invested heavily in the experimental analysis that produces reliable individual treatment plans.

**The functional analysis of self-injury is a well-established procedure with a large literature.** The proportion of cases in which the behaviour is maintained by automatic reinforcement is consistently reported to be high, and this is the single most clinically consequential finding in the field.

**Function-based treatment plans are more effective than non-function-based plans**, and this is the finding that the whole approach rests on.

**Differential reinforcement, differential reinforcement of compliance, and planned ignoring are effective for the escape- and attention-maintained cases**, and the evidence for the treatment of aggression maintained by escape is good.

**Pharmacological management is effective for a subset of self-injury**, particularly in the specific genetic syndromes, and it is essential in those cases. The clinician who works only behaviourally will miss the treatable neurochemical cases.

**The evidence for the environmental and medical components is weaker**, largely because these are difficult to evaluate in controlled trials, and the field's assessment of them rests on the clinical logic that a change in the antecedents precedes a change in the behaviour.

**The evidence on restrictive interventions is substantial and it is consistently negative**, with the studies showing higher rates of injury, higher rates of subsequent aggression, and worse outcomes. This is the point at which the evidence and the ethics converge: the interventions that are used most often in the settings where this work happens are the interventions with the worst evidence and the greatest risk.

## 8.2 Limitations of the evidence

**The samples are small and highly selected**, and they are drawn disproportionately from the most severe cases, who are the least representative of the population.

**The outcome measures are almost exclusively the frequency of the behaviour**, and the measures of the young person's quality of life, of their sense of dignity, of the family relationships, and of the durability of the change in naturalistic settings are much less systematically reported.

**The designs are the single-case experimental designs of Chapter 24**, and although they are rigorous, they are vulnerable to the reactivity of the participants and to the observer expectancy that the field has worked hard to address.

**The long-term outcomes are under-researched**, and there is a serious question about what happens to the young people whose behaviour has been reduced: what becomes of them, what is the placement plan, and what is the plan for the transition to adulthood.

**The literature is dominated by English-language research in North American and European settings**, and the cultural specificities of the settings in which this work happens — the tolerance for certain behaviours in a community, the family's understanding of the behaviour, the role of the extended family —

are under-examined.

# 9. Strengths and Limitations of the Functional Approach to Dangerous Behaviour

## 9.1 Strengths

**It is the only approach with a demonstrated mechanism.** The treatment is targeted at the contingency that maintains the behaviour, and when the contingency is identified and altered, the behaviour changes.

**It is the only approach that produces a differentiated plan.** A young person whose self-injury is automatically maintained and a young person whose aggression is escape-maintained receive entirely different plans, and the analysis is what makes the difference visible.

**It reduces the reliance on the most restrictive interventions**, because the analysis identifies the environmental changes that remove the need for them.

**It produces a written, teachable, and auditable plan**, which is what a team, a family, and a regulator can all work with.

**It treats the young person's behaviour as information rather than as a moral failure**, which is the foundation of every intervention in this chapter.

## 9.2 Limitations

**The analysis is slow and the behaviour is urgent.** The experimental analysis of self-injury takes days to weeks, and in the interval the young person is at risk. The safety-first framework is the solution, and it is an imperfect one.

**The analysis requires a stable setting with trained staff**, and the population in which this work is needed is characterised by instability and turnover. A plan written in one placement is frequently irrelevant in the next.

**The analysis can be wrong, and it can be wrong in a way that is hard to detect.** A behaviour that is maintained by several functions may appear to respond to one, and the response may be partial.

**The treatment of the behaviour is not the treatment of the young person.** A young person whose self-injury is a symptom of psychosis requires the treatment of the psychosis, and a young person whose aggression is a symptom of a depressive episode requires the treatment of the depression, and the behavioural plan is an adjunct. The functional approach is powerful precisely because it can be applied to behaviour that has no other treatment, and this is also its limitation, because it can be applied to behaviour that does have one.

**The approach requires an institutional commitment that many settings do not have.** The analysis is only as good as the environment in which the plan is implemented, and a placement that wants the behaviour to stop without wanting to change its environment will produce a plan that will fail.

# 10. Cultural and Ethical Considerations

## 10.1 The cultural frame of dangerous behaviour

**The definition of dangerous is culturally and legally specific.** What is considered a serious risk, what is considered an acceptable level of self-injury, and what the state is willing to do about it vary considerably between jurisdictions and between communities, and a practitioner must know the local legal framework, the local threshold for intervention, and the local understanding of the young person and the family.

**The meaning of the behaviour is culturally framed.** A young person's aggression towards a family member may be read in one community as a symptom, in another as a discipline that has been applied to her, and in a third as a form of protest. The practitioner who applies a functional analysis without asking the family what they understand the behaviour to be doing will design a plan that is fighting the family's understanding.

**The family's role is culturally variable.** In some settings the family is the treatment team; in others, the family is a source of additional risk; and in a substantial number of cases the family is the placement that the behaviour has already destroyed, and the practitioner must be able to work with a family that has been told that their child is dangerous.

**The stigma attached to the behaviour is severe and it is socially reproduced.** A young person with a record of violent behaviour or of self-harm is excluded from ordinary opportunities long after the behaviour has stopped, and the practitioner who reduces the behaviour and does nothing about the consequences of the record has delivered a treatment with a short-term benefit and a long-term harm.

## 10.2 Ethical requirements

**The safety of the young person is the first consideration**, and the safety of others is the second, and both are treated as absolute in the immediate term. The rest of the ethical requirements operate within that frame.

**The least restrictive effective procedure.** Every plan should be able to answer the question of what alternatives were considered and why they were rejected, and the answer should be written down. This is both an ethical and a legal requirement in most jurisdictions.

**Proportionality and the documentation of restrictive interventions.** Every use of restraint, of seclusion, and of any restrictive medication must be documented with the specific circumstances, the duration, the less restrictive options that were attempted, the injuries sustained by anyone, and the review that followed. A plan that does not document these is a plan that cannot be defended.

**The young person's dignity.** The interventions that protect a young person can be experienced as punishment, and the practitioner should name this in the plan and design against it. Three specific practices help: the young person is told what will happen before it happens, and the telling is not conditional on their cooperation; the interventions are applied with as little force and as little audience as possible; and the young person is involved in the review of the plan, which is the single most effective protection against the accumulation of unexamined restrictions.

**The right to be heard.** The young person has a right to be consulted about the plan, and to have their account of the behaviour recorded, and — in most jurisdictions — a right to complain about the interventions used. The presence of a complaints route in the plan is itself a protective practice.

**The right to the least restrictive environment.** The plan should specify the target environment, and the goal should be stated as the young person's return to the least restrictive setting in which the behaviour can be managed, with the steps and the criteria. A plan that has no stated goal of restoration is a plan of permanent containment.

**Family inclusion.** The family should be included in the plan, supported, and given something to do, and the support should be practical. The alternative is a family that is excluded from the plan and is then unable to support it.

**Coordination of the multidisciplinary team.** The plan should be developed by a team, reviewed by a team, and shared with the family, and a practitioner working alone in this area is working outside the professional standard.

**Avoidance of the instrumentalisation of the young person.** A young person whose dangerous behaviour makes them valuable as a demonstration, as a training case, or as a research participant has been abused, and the field's history contains a number of such episodes.

## 11. Key Terms

### Key Terms at a Glance

- **Self-injurious behaviour (SIB)** — Behaviour in which a person inflicts physical harm on themselves, including skin picking, scratching, cutting, head banging, eye poking, and hair pulling.
- **Aggression** — Behaviour intended to cause harm, damage, or intimidation to another person or to property; including verbal aggression, destruction, and physical violence.
- **The safety-first framework** — The ordering of the clinical work in which immediate safety precedes medical assessment, which precedes the functional assessment, which precedes the treatment.
- **Safety plan** — The written, shared, and rehearsed specification of the predictors of the behaviour, the response at each stage, the provision of the alternative, and the response after an episode.
- **Early indicators** — The physiological and behavioural precursors of an episode, individual to the young person, and usually the most valuable single output of the descriptive assessment.
- **Functional analysis of self-injury** — The experimental determination of the consequence that maintains the behaviour, using alone, play, attention, demand, and control conditions.
- **The alone condition** — The experimental condition in which the young person is alone with an observer, used to identify automatically maintained behaviour.
- **The play condition** — The condition in which the environment is arranged to be engaging and the behaviour is not attended to, used as the control for automatic maintenance.
- **The attention condition** — The condition in which attention is delivered contingently on the behaviour, used to identify attention-maintained behaviour.
- **The demand condition** — The condition in which demands are made and escape is contingent on the behaviour, used to identify escape-maintained behaviour.
- **The control condition** — The condition in which the consequence of interest is delivered non-contingently, controlling for the presence of the consequence rather than its contingency.
- **Automatic reinforcement** — Maintenance of a behaviour by its own sensory or affective consequences, without reference to the social environment; the most common function of self-injury.
- **Automatically maintained behaviour** — Behaviour that persists in the alone and play conditions and is not maintained by a social consequence.
- **Escape-maintained aggression** — Aggression that reliably terminates or postpones a demand; the most common function of aggression in young people and the most reliably treated.

### Key Terms at a Glance (continued)

- **Attention-maintained aggression** — Aggression that reliably produces adult attention; the treatment is differential reinforcement of the appropriate behaviour and the planned ignoring of the aggression.
- **Differential reinforcement of compliance (DRC)** — The reinforcement of compliance with a demand together with the extinction of the escalation; the core treatment for escape-maintained aggression.
- **Planned ignoring** — The deliberate and complete non-response to a behaviour, with no eye contact, speech, or signal, used to extinguish attention-maintained behaviour.
- **The escalation sequence** — The detailed record of the adult behaviour and the young person's behaviour during an escalation; usually the most informative single component of the analysis of aggression.
- **Establishing conditions** — The historical circumstances in which the behaviour was first performed and acquired its function.
- **Setting demands** — The automatic, attention, escape, and combined demands describing the circumstances under which the behaviour occurs.
- **The behavioural alternative** — A behaviour that is functionally equivalent to the target behaviour and is more efficient, more acceptable, and less dangerous.
- **Extinction burst** — A temporary increase in the frequency or intensity of behaviour at the start of extinction, which is expected and should be predicted to the young person and the team.
- **Restrictive intervention** — Physical restraint, seclusion, or the chemical equivalent; a last-resort intervention of shortest possible duration.
- **Last-resort intervention** — An intervention used only when the young person poses an imminent danger and less restrictive measures have been tried and failed.
- **Manifestation determination** — The procedural requirement, under disability legislation, that a behaviour resulting in a change of placement be assessed against the possibility that it is a manifestation of the young person's disability.
- **Functional behavioural assessment (FBA)** — The assessment required in many jurisdictions before a change of placement for a student with a disability.
- **Medical review** — The systematic review of medication, pain, sleep, seizures, nutrition, and the sensory environment, conducted before the behavioural analysis of a dangerous behaviour.
- **The mood-disorder presentation of aggression** — Irritability as the characteristic presentation of a depressive episode in adolescence, frequently misreferred as aggression.

**Key Terms at a Glance (continued)**

- **Lesch–Nyhan syndrome** — A rare genetic disorder associated with a characteristic pattern of severe self-injury and with a neurochemical profile that is substantially responsive to pharmacological management.
- **Corneal self-injury in Smith–Magenis syndrome** — A characteristic and severe self-injurious behaviour associated with a specific genetic syndrome and with a distinctive sleep disturbance.
- **Dignity protection** — The clinical practice of designing safety interventions so that they are not experienced by the young person as punishment.
- **Restoration goal** — The explicit target of returning the young person to the least restrictive environment in which the behaviour can be managed.

## 12. Chapter Summary

This chapter treated two behavioural presentations that are severe enough to require a different set of clinical conventions from the rest of the book, and it stated the safety-first framework at the outset: the safety of the young person comes first and is not contingent on the completeness of the assessment; the least restrictive effective procedure governs the intervention; the treatment of the behaviour is not the treatment of the condition; and the intervention must be accompanied by a plan for the young person's dignity.

The history explained why the field arrived where it has: the institutional tradition treated unexplained behaviour harshly, and the decisive development was the experimental analysis of self-injury, whose central finding — that the behaviour is overwhelmingly maintained by automatic reinforcement — changed practice fundamentally, because containment removes the behaviour without removing the contingency that produces it. The parallel work on aggression established that much of it is escape-maintained, which is the direct opposite of the assumption embedded in most schools' responses.

The safety hierarchy was set out at four levels, and the safety plans for self-injury and for aggression were described in their five components each, with the escalation procedure and the return condition identified as the most important and the most frequently improvised elements. The position on restrictive interventions was stated plainly: last resort, shortest duration, with oversight, and associated with higher rates of injury and worse outcomes, which is an argument for the seriousness of their use and for the institutional responsibility to design them out of the environment.

The clinical presentations were described with their differential diagnoses, and two specific points were emphasised: the genetic syndromes in which pharmacological management is a substantial part of the treatment, and the mood-disorder presentation of aggression in adolescence, which is frequently misreferred to a behaviour service. The assessment was described as a comprehensive exercise, with the risk assessment identified as a separate document with a separate structure.

The functional analyses of both behaviours were described in detail, with the conditions, the interpretation, and the establishing conditions, and the treatment of the escape-maintained case of aggression was set out in the detail in which it must be implemented, including the predicted extinction burst and the requirement for the adults' response to the escalation to be specified, trained, and consistent.

The case of Marcus demonstrated the sequence in operation and, in doing so, demonstrated the limits of the method. The experimental analysis found the automatic function and the treatment addressed it, and the young person's own account, offered in the eighth week, named the escape function that the plan had not addressed. Both were correct, and the argument of the case is that the young person's own account is data, is gathered as early as possible, and is a supplement to rather than an alternative to the experimental analysis.

The evidence is methodologically mature for the functional analysis and for function-based treatment, and the evidence on restrictive interventions is consistently negative. The limitations were stated: the small and highly selected samples, the behavioural rather than experiential outcome measures, the single-case designs, the under-researched long-term outcomes, and the cultural narrowness of the literature.

The ethical requirements were set out in full — least restrictive procedure with documented alternatives, proportional and documented restrictive interventions, dignity protection with three specific practices, the right to be heard and to complain, the right to the least restrictive environment with a restoration goal, family inclusion with practical support, team coordination, and the refusal to instrumentalise the young person. The cultural analysis addressed the culturally and legally specific definition of danger, the culturally framed meaning of the behaviour, the variable role of the family, and the severe social stigma that follows the young person long after the behaviour has stopped.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. State the four elements of the safety-first framework and explain why the order is not negotiable.
2. Describe the four levels of the safety hierarchy and the work that belongs at each.
3. State the five components of a safety plan for self-injury.
4. State the five components of a safety plan for aggression, and identify the one that differs in kind.
5. Explain the role of the early indicators and why they are the most valuable output of the descriptive assessment.
6. State the four clauses of the professional position on restrictive interventions, and summarise the evidence on their use.
7. Describe the differential diagnosis of self-injurious behaviour, and state the clinical consequence of each major category.
8. Describe the differential diagnosis of aggression, and explain why the mood-disorder presentation is frequently missed.
9. State the six components of the assessment of dangerous behaviour, and explain why the risk assessment is a separate document.
10. State the four stages of the functional analysis of self-injury and explain the order.
11. Describe the six experimental conditions and state what each is designed to detect.
12. Explain the difference between the function, the setting demands, and the establishing conditions.
13. Explain the three modifications required in the functional analysis of aggression.
14. Describe the six steps of the treatment of the escape-maintained case of aggression.
15. Explain the role of the density of the reinforcement schedule in the early phase of differential reinforcement of compliance.
16. Explain why the plan for the aftermath of an aggressive episode is a clinical task and not a courtesy.
17. Describe the case of Marcus and state the two arguments it makes about the method.
18. Summarise the status of the evidence for the functional analysis of self-injury and for function-based treatment.
19. Summarise the evidence on restrictive interventions and explain its clinical and ethical implications.
20. State the five limitations of the evidence base.
21. State the five strengths and the five limitations of the functional approach to dangerous behaviour.

**Review and Discussion Questions (continued)**

22. Explain the four cultural considerations that bear on this work.
23. State the ten ethical requirements for work with dangerous behaviour, and explain the significance of the documentation requirement.
24. Explain the three specific practices that protect the young person's dignity.
25. Explain the significance of the right to the least restrictive environment and of the restoration goal.

## 14. Practice Activities

**Practice and Application**

### Activity 29.1 — The Safety Plan (90 minutes, individual)

Write a complete safety plan for a fictional young person with a behaviour that is dangerous to themselves. Specify the functional prediction, the early indicators, the response to the early indicators, the response to the behaviour, the provision of the alternative, and the response after an episode. Then write the specific sentence that an adult will say to the young person at each of the three points. The sentences are the hard part, and the exercise produces a plan that is genuinely usable rather than a document that satisfies a template.

### Activity 29.2 — The ABC of a Dangerous Behaviour (75 minutes, field-based)

With supervision, record the antecedents, the behaviour, and the consequences of a dangerous behaviour in a placement, using the detailed record rather than the summary one. Record in particular what the adult did immediately after the escalation, because the escalation sequence is usually the most informative component of the whole assessment. Then write the two competing functional hypotheses and the observation that would distinguish them. Do not intervene in the escalation, and do not record information that identifies the young person to anyone who does not need it.

### Activity 29.3 — Restorative versus Extinction (60 minutes, small group)

In groups of three, take a behaviour maintained by escape and design two plans: one that uses differential reinforcement of compliance with extinction of the escalation, and one that uses a restorative approach in which the young person repairs the consequence of the escalation. Identify what each plan predicts will happen in the first two weeks, and identify what each plan requires of the adults. Then explain why a plan that mixes the two approaches is usually worse than either. The discussion of the adults' requirements is where the exercise earns its value.

### Activity 29.4 — The Dignity Audit (45 minutes, individual)

Take a safety plan for a young person, real or fictional, and audit it from the young person's point of view. For each element, write what the young person will think and feel when it happens. Then revise the plan so that the young person's experience of it is defensible. The revisions are usually small — the timing of a warning, the choice of a location, the wording of a sentence, the presence of an audience — and the sum of them is the difference between a plan the young person co-operates with and one they will not.

## 15. Annotated Reading Pathways

**For the method.** Read Iwata and colleagues (1982) and then the 1994 book, and read the chapter on the functional analysis of self-injury in any current applied behaviour analysis textbook. The 1982 paper is short and is still the best introduction to the logic of the experimental analysis of dangerous behaviour.

**For the treatment.** Read Aman and colleagues (2000) and the current treatment reviews, and read the population-specific chapters on autism spectrum disorder and on the genetic syndromes. The pharmacological section is not optional in those populations.

**For the safety and the law.** Read the local regulations on seclusion and restraint, and read the professional position statements. Read them before working in the area, not after an incident.

**For the evidence.** Read the reviews of function-based treatment and the studies of restrictive interventions, which are consistently negative and which are the most important empirical basis for the ethical position in this chapter.

**For the ethical and cultural analysis.** Read the disability rights literature on the treatment of people who are dangerous and on the rights of people with intellectual disabilities, and read the literature on the social exclusion that follows a history of violent behaviour. The combination is the preparation for the part of the work that happens after the behaviour has stopped.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Functional Analysis of Self-Injurious and Aggressive Behaviour** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is safety-oriented assessment and intervention with behaviour that is harmful or dangerous. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The mechanism to keep in view is how antecedents, consequences, skills, avoidance, and environmental design select and maintain behaviour. The characteristic professional risk is using control without assent, reinforcing compliance rather than wellbeing, or ignoring the meaning and function of behaviour. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in classrooms, homes, clinics, and other real environments where behaviour occurs, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Sofia is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Sofia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Sofia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through

repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on functional analysis, learning history, contingencies, mastery, and measurable behaviour change. The first intervention is deliberately modest. Sofia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Sofia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Sofia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Functional Analysis of Self-Injurious and Aggressive Behaviour

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Functional Analysis of Self-Injurious and Aggressive Behaviour** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Functional Analysis of Self-Injurious and Aggressive Behaviour** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Functional Analysis of Self-Injurious and Aggressive Behaviour**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Functional Analysis of Self-Injurious and Aggressive Behaviour to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART V

# Systemic, Family, and Relational Theories

Theories that locate the individual inside a network of relationships and meanings: the systemic turn, the family as the unit of treatment, and the narrative and solution-oriented traditions.

## CHAPTERS IN THIS PART

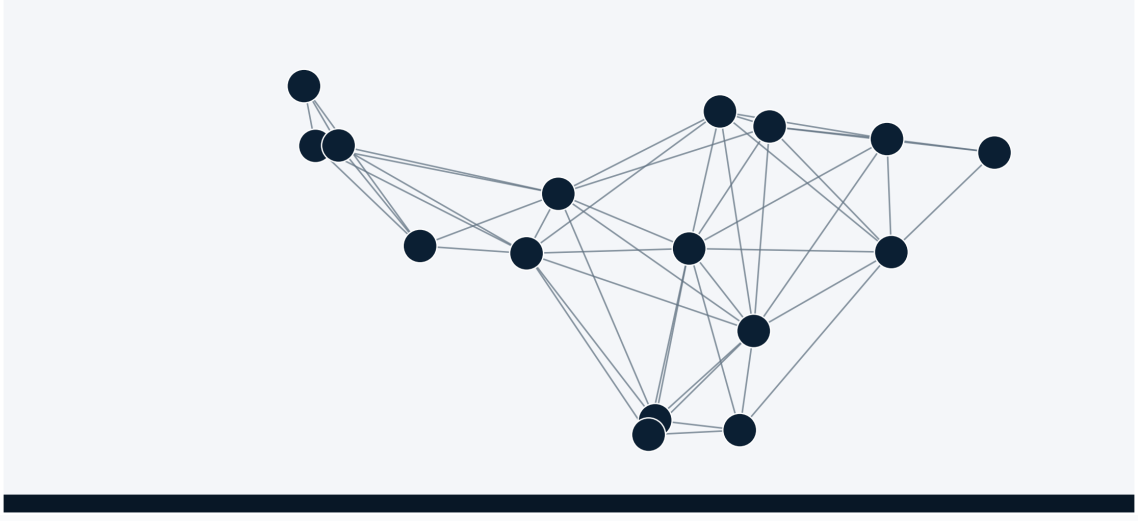

30 Family Systems Theory: Bowen and the Generational Project
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35 Solution-Focused Brief Therapy and Strength-Based Practice
36 Narrative Therapy and the Re-Authoring of Lives




CHAPTER 30

# Family Systems Theory: Bowen and the Generational Project

Differentiation, triangles, nuclear family emotional process, and the genogram as a clinical tool.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of the family systems tradition, from the early family therapy movement to Bowen's work and its descendants.
- State the eight core concepts of Bowen's family systems theory and explain the clinical significance of each.
- Explain the concept of differentiation of self, and distinguish it from individuation, autonomy, and emotional detachment.
- Describe the formation and function of triangles, and explain why almost all clinical material with a young person is triadic.
- Describe the nuclear family emotional process, including the concept of the emotional cutoff and the generation of the family projection process.
- Construct a genogram, and explain its uses in assessment and in intervention.
- Apply the Bowen framework to a case involving an adolescent and a family, and evaluate the limits of the framework.
- Critically evaluate the cultural and ethical dimensions of generational family systems work.

## Chapter Overview

Bowen's family systems theory is unusual in the counselling field in two respects that a student should appreciate before reading the theory.

The first is that it is a **theory of the whole of a person's life in a small number of concentric circles**. Bowen did not treat the family as one system among many, nor as a context for an individual problem. He treated the family as the fundamental unit of emotional life, and he described the person's functioning as a series of nested systems — the two-person dyads, the triangles, the family subsystems, the family, the extended family, and the wider community and culture — each of which is governed by the same processes at a different scale. A young person's symptom is therefore not a disorder located in the individual but a symptom of a disturbance in the process of the family, and it can be read at every level simultaneously.

The second is that it is a **theory with a clinical method attached to the assessment**. Bowen developed the genogram — the family diagram — as a clinical instrument, and his use of it to trace the patterns of relationship across three or four generations is the most distinctive contribution of the tradition to the practice of counselling. The genogram does something that no other clinical tool does: it makes the intergenerational dimension of a young person's difficulty visible, and it makes it visible in a form that a family can look at together.

The theory's weaknesses are as well known as its strengths, and they should be understood before the theory is applied. Bowen did not conduct controlled outcome research, and the evidence base for the theory as a whole is weak, with the evidence concentrated on specific applications such as family-based treatment of eating disorders and of substance use, where the family component has a demonstrable effect. The theory's assumption that the family is the fundamental unit of emotional life does not hold in the same way for every young person, and the account of the family as a system of emotional processes has been criticised for a tendency to locate the problem in the family when the problem has been located in the wider social

structure.

This chapter presents the theory in its technical form, the genogram as a clinical method, and the adaptations that the work with young people requires.

# 1. Historical Context

## 1.1 The origins of family therapy

Family therapy emerged in the United States in the 1950s, and it emerged because of a clinical problem that the individual model could not solve: a substantial number of patients improved in individual therapy and then relapsed as soon as they returned to their families. The observation was so consistent that it forced a reorientation, and the reorientation took the form of a simple and radical proposition — that the family, not the individual, is the unit of treatment, and that the symptom is a function of the family's structure and its patterns of interaction.

The early work was conducted in two settings that were quite unlike any contemporary mental health service. The first was the **child guidance clinics**, where the child was understood to be the identified patient and the family was understood to be the context; the second was the **institutions**, where the deinstitutionalisation movement exposed the extent to which chronic disturbance was maintained by the institutional structure itself.

The field's founding figures were diverse in background and in method, and the diversity is instructive. **Murray Bowen**, a psychiatrist and psychoanalyst, developed the family systems theory and the genogram. **Virginia Satir**, a social worker, developed a humanistic and skills-oriented family therapy. **Salvador Minuchin**, a psychiatrist, developed the structural model. **Jay Haley**, a communications specialist, developed the strategic and communication approaches. **Carl Whitaker**, an experiential therapist, developed a distinctive and influential model of the family as a developmental context.

The division that emerged between the "structural" and "strategic" camps, and later between Bowen and the others, is partly a matter of theory and partly a matter of professional temperament, and a student should know that the family therapy field has never been a unified discipline.

## 1.2 Bowen and the generation of the idea

Bowen's training was in psychoanalysis, and he was in the unusual position of being able to see the limits of the individual model from within it. His central clinical observation was that his analysands' families reproduced their symptoms in the session: a patient who was withdrawn in the consulting room had a family that was warm and intrusive in the family room, and a patient who was anxiously enmeshed had a family that was cool and distant at home.

The implication was that the anxiety was not located in the individual but was a *product of the relationships*, and that the individual patient's symptom was a consequence of the position he occupied in the family system. This is the shift from intrapsychic to interpersonal causation, and it is the shift that the entire family systems tradition rests on.

Bowen's theoretical contribution was to make the interpersonal shift *quantitative* and *generational*. The family was not a single system; it was a nested set of systems governed by the same processes at different scales, and the processes had a lineage that ran through the generations.

### 1.3 The development of the tradition

Bowen's own practice was unusual. He worked with families of varying composition, he used the genogram as a routine part of assessment, he was interested in the transmission of emotional processes across generations, and he formulated in a language that was intended to be shared with the family rather than concealed from it. He was also, characteristically, cautious about the ambition of the systemic work: he described his approach as a way of *thinking about* a family rather than a technique for *treating* one.

The descendants of the tradition are numerous. **Murray Bowen, Jr.** and the group at the Family Institute in Washington developed Bowenian family therapy and the clinical training that carries his work forward. **Carl Whitaker's** experiential family therapy, developed in parallel, is distinctive in its attention to the emotional experience of the family members and in its refusal to take the identified patient's account at face value. **Murray Bowen's** articulation also informed the **structural** and **strategic** schools, which are the subjects of Chapters 31 and 32.

### 1.4 The reception

Two distinct lines of reception are important for a student.

**The clinical reception.** Bowenian family therapy is widely used in academic training and in a minority of clinical settings, and it is influential well beyond its direct practice, because the genogram is now a standard tool in assessment in family therapy, in safeguarding, in school counselling, and in adoption and fostering practice.

**The critical reception.** The theory has been criticised on several grounds, and each is worth stating because each identifies a real limitation.

**It is empirically thin.** Bowen did not conduct controlled outcome research, and the research literature on Bowenian family therapy is small. Where family-based approaches have been shown to work, the demonstrations are usually of components — family-based treatment of adolescent eating disorders, family therapy for substance use, behavioural family therapy for conduct problems — rather than of Bowen's theory as a whole.

**It risks a circularity.** If a young person's difficulty is understood as a product of the family, and the family's patterns are understood as the products of earlier generations, and the earlier generations are understood in the same terms, then the young person can end up as the tip of a lineage rather than as a person, and the clinical work can end up directed at the ancestors.

**It under-accounts for the structural.** The theory treats the family as a system of emotional processes, and it can under-specify the material and political realities within which the family lives: poverty, racism, migration, unemployment, housing, and the effects of state decisions on the family's capacity to function. A formulation that attributes a family's difficulties to its internal emotional process risks locating the problem in the family when the problem has been located in the conditions.

**It assumes a family.** The theory presupposes a coherent and continuing family system. For a young person in care, for a young person separated from parents, for a young person in a family that has been dissolved by migration, conflict, or violence, the presupposition does not hold, and the practitioner must adapt the theory substantially or work with a different one.

## 2. The Systems Perspective

## 2.1 The eight core concepts

Bowen's family systems theory is conventionally described in terms of eight core concepts, and a student should be able to state all eight and give an example of each in a family with a young person.

1. **Two-person systems**, in which the relationship between two individuals is the primary unit of emotional life.
2. **Differentiation of self**, the degree to which a person can maintain a clear sense of self within an emotional relationship.
3. **Triangles**, the three-person systems that form when the tension between two people is relieved by the involvement of a third.
4. **Nuclear family emotional process**, the transfer of emotional process from one generation to the next, transmitted through the parental subsystem.
5. **Emotional cutoff**, the process by which a family member distances from the family in order to avoid anxiety, and the transmission of that cutoff across generations.
6. **The multigenerational transmission process**, the description of how emotional processes are transmitted across generations.
7. **The family projection process**, the mechanism by which the parental subsystem transmits its own emotional process to a child.
8. **Sibling position**, the way in which a child's position in the birth order shapes the family's emotional process.

Two further concepts, the **sociocultural emotional process** and the **family emotional system**, complete the framework. The sociocultural process describes the way in which the emotional processes of a family are shaped by the wider culture, and it is the concept through which the theory can be brought to bear on the structural questions that the theory otherwise under-specifies.

## 2.2 The nested systems

The defining feature of the framework is that it is **scale-invariant**: the same processes operate at every level of the system.

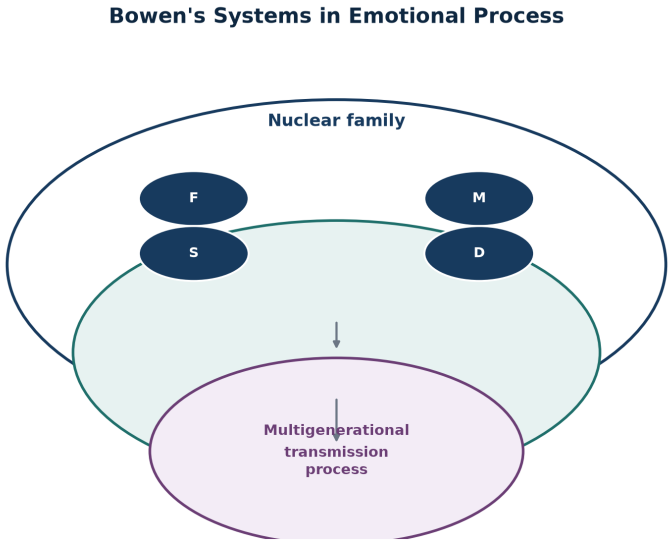



**Figure 30.17** Bowen's nested systems, showing the concentric levels from the two-person dyad through the triangle and the family subsystems to the wider family, the community, and the culture.

At the level of the **two-person system**, the fundamental emotional reality of the human being is the capacity to relate to another. Bowen was explicit that this is a level of *emotional* relationship and not of *rational* contract, and the distinction matters: a person can be entirely clear about the terms of a relationship and entirely unable to maintain a defined self within it.

At the level of the **triangle**, tension between two people is relieved by the involvement of a third, and the two sides of the triangle are in a state of tension with each other while the third is in a state of tension with both. Almost all emotional conflict in families involves triangles, and the practitioner who cannot identify the triangle will not understand the case.

At the level of the **family**, the sub-systems — the parental, the sibling, and the individual — are pulled towards a lower level of differentiation under stress, and the symptom is the mechanism by which the tension is held in the system.

At the level of the **extended family and the community**, the processes continue, and the community and the culture become the wider contexts in which the family negotiates.

## 2.3 The governing laws

The framework is described by two governing laws, and they are the most quoted and least understood part of it.

**Law one: the preservation of self.** Bowen drew this from William James and from the philosophical literature, and it states that the organism is a whole and that the function of the whole is to preserve the whole — that is, the individual. The "self" in the law is a functional term: it refers to the capacity of the organism to maintain its integrity and to function as a whole.

**Law two: the preservation of the family** — or, in its common formulation, the emotional system of the family is more important than any individual member of it.

The two laws are in tension, and the tension is the source of the clinical problems the theory describes. Under stress, the family system pulls the individual towards the family (law two), and the individual resists (law one), and the visible result is a symptom in one of the members.

A student should be able to explain the laws and their tension, and should resist the interpretation — which is widespread in the popular literature — that the theory counsels self-sacrifice or family loyalty. It does not. It says that the pull towards the family and the push towards the self must be balanced in a way that permits both, and that the balance is called differentiation.

# 3. Differentiation of Self

## 3.1 The concept

**Differentiation of self** is the degree to which a person can maintain a sense of self within an emotional relationship — the capacity to think, to act, and to feel in a way that is determined by one's own convictions rather than by the emotional pressure of the relationship, while remaining fully engaged in the relationship.

Three elements are involved: the **thoughts**, which are not contaminated by the emotional pressure of the situation; the **feelings**, which are not overwhelmed by the pressure of the situation; and the **actions**, which are not determined by the pressure of the situation. The degree of differentiation is the degree to which these three are congruent with each other and with the person's own position.

The key phrase is *within an emotional relationship*. Differentiation is not the same as independence, and a person who leaves the family, or who refuses to feel anything, or who maintains a position at the cost of every relationship, is not differentiated. And it is not the same as emotional detachment, which is the avoidance of emotional involvement in order to protect the self.

## 3.2 The two extremes

The two poles of the continuum describe the two characteristic family positions.

**The undifferentiated position** is characterised by the flooding of thought, feeling, and action by the emotional process of the relationship. The person cannot think clearly in the presence of the conflict, cannot modulate the feeling, and acts impulsively or rigidly. In the family this position is often occupied by the person who is the most involved in the family's emotional process.

**The cut-off position** is characterised by the reduction of emotional involvement in order to preserve the self. The person thinks and acts clearly, but at the cost of feeling, and the feeling is often displaced into physical symptoms, into a narrow pursuit, or into the body. The cut-off is a *process* and not a position, and it is a process in motion, which is why the families of cut-off young people often describe an abrupt disconnection — a departure, a marriage, a collapse in contact.

Between the two poles lies the differentiated position, in which a person can be close and can be autonomous, can feel deeply and can think clearly, and can disagree and can remain in the relationship.

## 3.3 The clinical significance

Differentiation is the central clinical concept, and it operates at two levels: the intrapersonal and the interpersonal.

**Intrapersonal differentiation** is the difference between the person's various states. A differentiated person can recognise that the anxiety she feels about her mother's phone call is not the same thing as the anger she feels about the last thing her mother actually said, and can distinguish between the fact and the interpretation, and can act on the fact.

**Interpersonal differentiation** is the difference between the people in the relationship. A differentiated parent can hold two different views — "my daughter is suffering" and "my daughter is also refusing to see me" — without collapsing the one into the other, and can express both to the daughter at the same time.

The clinical targets follow directly. A family with a symptomatic young person typically contains at least one undifferentiated member, whose flooding is transmitted to the others, and typically contains at least one cut-off member, whose disconnection leaves the family to manage its own anxiety. The work is to increase the differentiation of the family, and the symptom is what will reduce as a by-product.

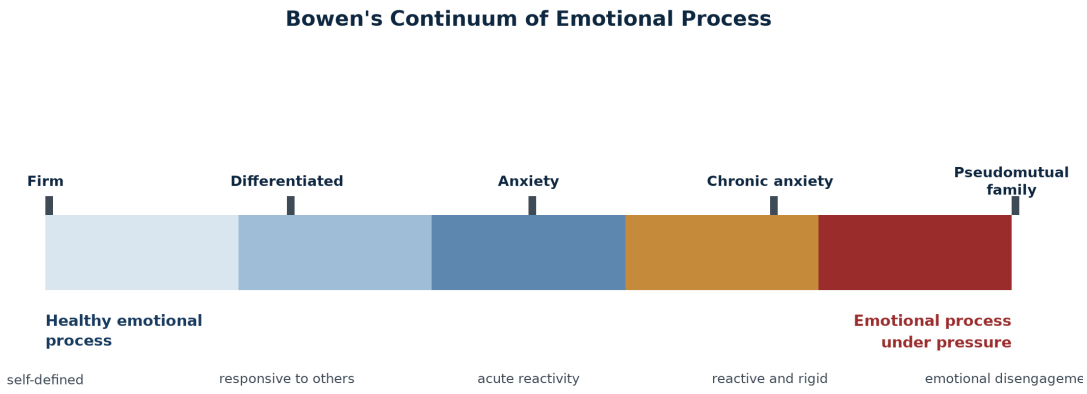



**Figure 30.18** The two poles of differentiation, showing the movement of thought, feeling, and action from the undifferentiated position through the differentiated position to the cut-off position.

## 3.4 Emotional process in the body

Bowen's clinical observation that the emotional process is transmitted through the body is one of the most immediately useful and least theoretical parts of the tradition, and it has a direct place in a book for practitioners who work with young people.

The observation is that family members' bodies synchronise in the presence of tension. Posture, breathing, heart rate, and muscle tension change together, and the change is observable in a session within minutes, and the practitioner can use it as a data source. Bowen wrote about the "body copy" of the family emotional process, and the observation is the ancestor of the somatic and interpersonal neurobiological work in Part VI.

For a practitioner, three practical consequences follow. The young person's somatic symptoms may be a direct expression of the family process, and the treatment of a recurrent unexplained physical complaint should include a look at the family system. The practitioner's own body is a diagnostic instrument, and the practitioner can learn to notice their own somatic response to a family member. And a family that is being asked to talk about its emotional process should be prepared for the somatic experience of doing so, and the practitioner should expect an increase in somatic presentation in the early sessions.

# 4. Triangles

## 4.1 The formation of a triangle

A **triangle** forms when the tension between two people is relieved by the involvement of a third, and it has two sides. In a **comfortable triangle**, the two sides are in tension with each other and the third person is in tension with both. In a **conflictful triangle**, the two sides are in tension with each other and the third person is in tension with one side and aligned with the other.

The transfer of tension into a triangle is a universal human process and it happens in the consulting room as well as in the home. A young person who is anxious about a parent's marriage will frequently become the focus of the parents' conflict, and the parental conflict will frequently stop when the young person becomes symptomatic, because the symptom has provided a third position that both parents can occupy.

## 4.2 The clinical importance of triangles

**Almost all emotional conflict is triadic.** A practitioner who works with the identified patient alone is working with one vertex of a triangle and has no information about the other two. This is a practical argument for including the family, and it is an argument the theory makes without any appeal to family loyalty.

**The symptomatic member is usually the most involved.** In a family with a symptomatic young person, the young person is frequently the most enmeshed, and the parents are frequently the more differentiated, and the pull towards the family is what produces the symptom.

**Every triangle must be tracked.** The practitioner working with a family learns to ask, of every conflict between two people, "who is the third?" — and the answer, when it is found, explains the conflict that would otherwise be puzzling.

**The symptom stabilises the triangle.** This is the crucial clinical point, and it explains the family's resistance to change. The symptom is what holds the tension in the family at a manageable level, and removing it without substituting another structure releases the tension, and the released tension is experienced by the family as a catastrophe. A practitioner who removes a symptom quickly and does not replace the triangle structure has produced a worse outcome, and the family will describe the change as an escalation.

The clinical implication is that a plan for a young person whose symptom is doing structural work must include a plan for the family.

## 4.3 The "just" relationship

A specific and useful clinical distinction is between the **intense** and the **"just" relationship**. In the just relationship, the people involved maintain contact and mutual support while keeping the emotional intensity low. A young person who is in a just relationship with a parent continues to see the parent, continues to talk to the parent, and does so without the fusion and the conflict that characterise the intense relationship.

The just relationship is the target of most systemic work with a young person: not the resolution of the conflict but the reduction of the intensity, and the establishment of a workable low-intensity contact.

# 5. The Nuclear Family Emotional Process

## 5.1 The transmission

**Nuclear family emotional process** is the description of the way in which the emotional process of the parental subsystem is transmitted to the children, and it is the mechanism by which the intergenerational transmission occurs.

The transmission is not of content but of **process**. What is transmitted is the *way of handling emotional tension* — the level of differentiation, the capacity to sustain anxiety, the pattern of triangulation, the response to conflict. A parent who cannot tolerate anxiety and who resolves it by triangulation transmits the capacity to tolerate anxiety to the children, and the children develop the same intolerance, and the intolerance is expressed in the next generation as a symptom.

## 5.2 Emotional cutoff

**Emotional cutoff** is the process by which a family member reduces emotional involvement with the family in order to reduce anxiety, and it is the most common response to a family system that is too intense for the individual to bear.

It is worth distinguishing three forms, because they have different clinical consequences. The **healthy cutoff** is a psychological separation that permits the individual to function in a family that cannot be changed; the young person who goes to boarding school and maintains a warm relationship with the parents has made a healthy cutoff. The **internal cutoff** occurs within the family, in which the person remains in contact but ceases to be emotionally available; this is the young person who is physically present and psychologically elsewhere. The **external cutoff** is a physical or social severance; this is the young person who is removed from the family or who removes themself.

## 5.3 The family projection process

The **family projection process** is the mechanism by which the parental subsystem transmits its own emotional process to a child, and it is the specific mechanism that makes the transmission intelligible.

The process has two linked components.

**The projection** is the transmission of the parents' own unresolved emotional issues, attitudes, and emotional needs to the child. The parent projects onto the child a part of their own emotional world, and the child is shaped by the projection.

**The introjection** is the child's absorption of the projected material, which becomes part of the child's own emotional structure. A child who has been the object of a parent's projection of their own anxiety and their own unmet needs will internalise that anxiety and those needs, and will grow up with the sense of being responsible for the regulation of someone else's emotional state.

The clinical significance is the explanation it provides for the young person who is over-responsible, who is the family's emotional caretaker, who cannot let anyone else worry, and who finds it impossible to be in need. This is one of the most recognisable presentations in the family literature, and it is very often mistaken for a young person's character trait.

## 5.4 Sibling position

**Sibling position** — the order, the age gaps, the sex composition, and the number of children in the family — shapes the family emotional process. Bowen described the "sibling position" in terms of the child's place

in the birth order and the emotional characteristics associated with the position, and the descriptions are provocative and should be treated as hypotheses rather than as findings.

The useful clinical proposition is the one about the family projections: a family with two children tends to focus its projection on one of them, and the choice is often determined by the compatibility of the child's temperament with the parents' unresolved issues, and the child who is the focus of the projection is frequently the one who presents with the symptom, and frequently the one who is described as the "good" one.

The other useful proposition concerns the **family projection of a sibling position into the parental system**: a child who occupies the position of a parent's own sibling, and who resembles that parent, will receive a particular intensity of projection.

# 6. The Genogram

## 6.1 The instrument

The **genogram** is a family diagram, and it is Bowen's most widely used and most widely borrowed clinical contribution. It extends the pedigree — the standard family tree that traces biological descent — into a map of the *relationships* in the family across three or more generations.

The genogram records: the biological and the marital structure of the family; the names and the ages of the members; and, at a series of concentric levels, the social, psychological, medical, and emotional information about each member.

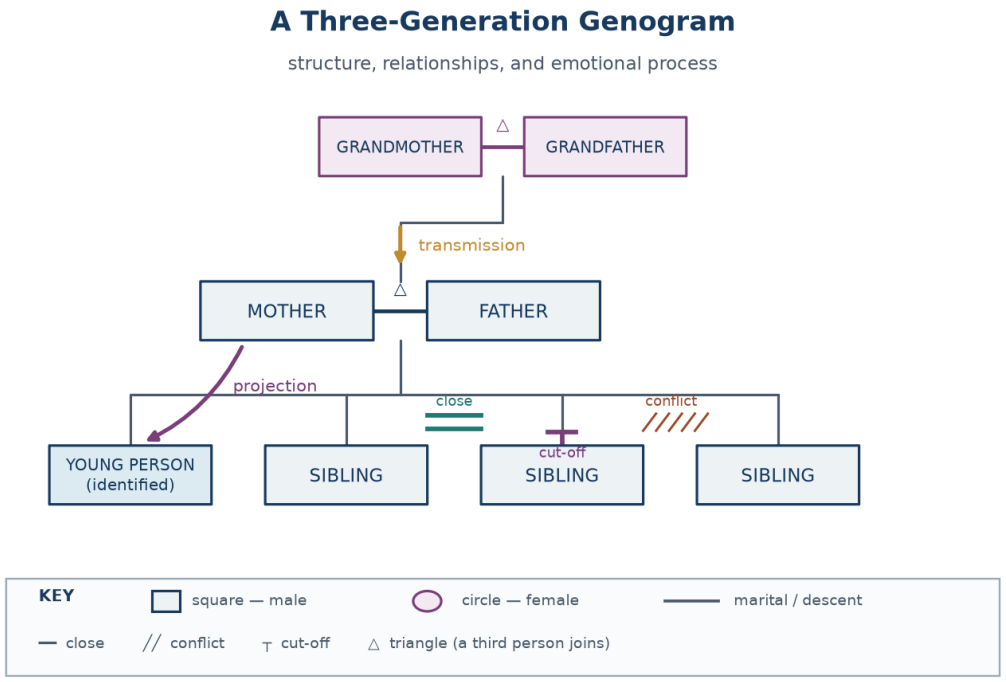



**Figure 30.19** A three-generation genogram showing the standard symbols for marital and descent lines, close and conflicted relationships, cut-off, and the triangle.

The levels are the key to the instrument. The **social** level records education, occupation, ethnicity, migration, and socio-economic status. The **psychological** level records the significant emotional events — the losses, the abuse, the mental illness, the substance use, the suicide attempts, the relationship ruptures — and the quality of the relationships between the members. The **medical** level records the illnesses, the genetic conditions, and the causes of death.

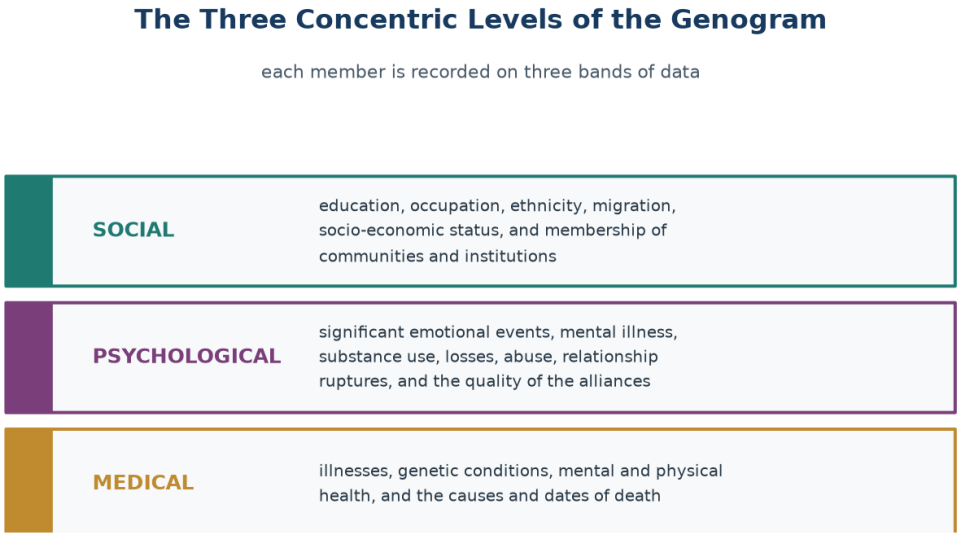



**Figure 30.20** The three concentric levels of the genogram, showing the social, psychological, and medical data recorded for each family member.

## 6.2 The procedure

The construction of a genogram is a clinical conversation, and it should be done with the family and not about them.

**The first session** establishes the structure: the marriages, the separations, the divorces, the children, the adoptions, the step-relations, the losses, the migration, and the family contacts that have been lost. The practitioner draws the genogram as the family speaks, because the drawing is itself a communication.

**The second session** adds the emotional level: the relationships, the alliances, the conflicts, the estrangements, the closeness. The symbols for these are the ones the family recognises immediately, and the genogram becomes a shared object that the family can see.

**The third session** adds the emotional process: the triangles, the cutoffs, the projections, the areas of intensity and of low intensity.

**The fourth session** addresses the family projection process, and the fifth the multigenerational transmission process.

## 6.3 The uses

The genogram is used for three distinct purposes, and it is worth separating them because they have different status.

**As an assessment instrument**, the genogram is of value in its own right. It is the most efficient way to discover the presence of a pattern that has been operating for three generations, and the discovery of such a pattern often reorganises the family's understanding of its own difficulty. A family that has always understood its difficulty as the fault of one member frequently re-describes it, after the genogram, as a long pattern.

**As a therapeutic instrument**, the genogram is used directly in the work. Bowen regarded the genogram as a way of placing the family in the context of its history, so that the current problem is seen as a legacy rather than as a personal failing, and the process of drawing it — with the family — is itself a form of intervention.

**As a supervision and consultation tool**, the genogram is used to make a case comprehensible to a team, and it is a far more efficient communication format than a narrative formulation.

### 6.4 The limits

Three limits should be named.

**It is a retrospective construction, and memory is unreliable.** The genogram is a family's account of its own history, and families' accounts are shaped by the current relationships. A grandmother's role in the transmission of the pattern may be remembered very differently by the grandmother and by the mother, and the disagreement is itself clinically interesting but it does not resolve the question.

**It can be a burden.** A young person who is shown a genogram that documents three generations of illness, loss, and conflict has been given a substantial amount to carry, and a young person who is handed the family pattern and told that it explains them may experience a loss of agency. The genogram should be used with the young person and not presented to them.

**It is not a research instrument.** Its reliability and validity for research purposes are poor, and a practitioner should not present a genogram as evidence.

## 7. Further Reading for This Chapter

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- Bowen, M. (1968). *The use of genograms in family therapy*. — The original paper on the instrument.
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- Shapiro, A. F. (2018). *The family in Bowenian theory*. — The clearest modern clinical exposition.
- Titelman, N. R. (2015). *Bowen family systems theory and its clinical applications*. — The clinical applications.
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# 8. Bowenian Work with Children and Adolescents

## 8.1 The fit

Bowenian thinking is unusually well suited to the developmental stage at which most clinical referrals of adolescents occur, and the reasons are worth stating before the clinical material.

**The adolescent's task is differentiation.** The developmental work of adolescence is precisely the work that Bowen's second core concept describes: the maintenance of a self within an emotional relationship, the increasing distance from the family of origin, the construction of a peer world that is not the family, and the eventual establishment of a life that is one's own. A theory whose central concept is the adolescent's central developmental task has an unusual advantage.

**The symptom has structural function.** The adolescent's symptom — the withdrawal, the defiance, the school refusal, the eating, the substance use — is very often doing the work of holding the family's emotional process in a manageable configuration, and a therapeutic approach that works only on the symptom will fail, because the family will re-establish the configuration.

**The triangle is the adolescent's natural habitat.** The adolescent's position between two parents is a developmental fact, and a framework in which the adolescent is understood as the third vertex of a parental triangle is more accurate than one in which the adolescent is understood as an individual with a disorder.

## 8.2 The clinical principles with young people

**Work with the family system, not only with the young person.** This is the theory's most distinctive practical claim, and it is also the one most often resisted. A practitioner who works only with the young person and then attempts to work with the family has usually missed the moment at which the family would have engaged.

**The genogram with the family.** The genogram is the most accessible of the systemic tools, and it works with young people particularly well because it gives the young person a place in a pattern that is larger than their own behaviour. A young person who has been told that their refusal to speak to their father is a character flaw is often relieved to discover that the pattern is a family one.

**Reduce the intensity rather than resolve the conflict.** The target is the *just relationship* — the maintenance of contact without the fusion. Many of the interventions that work with adolescents are simply the establishment of a different pattern of contact, and the establishment of a specific, small, regular contact is often enough to reduce the symptom.

**Watch for the cut-off as a symptom.** A young person who has stopped speaking to a parent is performing a cut-off, and the cut-off may be protecting them from an intolerable fusion, and the question is not how to make them speak but what contact would be tolerable.

**Do not make the young person the agent of change.** A common systemic error is to ask the young person to change in order to relieve the family's tension, which confirms the projection. The young person should not be the one whose behaviour must change so that the parents can feel better.

**Expect the family to resist.** The family's resistance is not an obstacle to the work; it is the material of the work. A family that resists a change has usually found a reason to resist it, and the reason is worth understanding.

### 8.3 The clinical sequence

A workable sequence for an adolescent referral in a Bowenian frame is as follows.

**Session 1: the individual.** An assessment session with the young person, focused on their own account, their own developmental position, and their own view of the family. The purpose is to establish the alliance and to hear the young person's account in their own terms, and the practitioner should resist the temptation to begin the systemic work here.

**Session 2: the genogram.** A session with the young person, or with the young person and one parent, in which the genogram is drawn. The young person is the primary source, and the genogram is drawn with the young person rather than about them.

**Session 3: the family.** The first family session, with the task of drawing the family emotional system, and of establishing the alliances and the conflicts. Bowenian practitioners are characteristically cautious about the intensity of early family work, and the caution is wise with an adolescent, because a first family session that becomes an argument between the parents will confirm the young person's position and drive them out of the process.

**Sessions 4 onward: the work.** The systemic work, most commonly the reduction of the intensity of the central triangle, the establishment of the just relationships, and the differentiation of the parental subsystem, with the work frequently proceeding through the parents rather than with the young person.

### 8.4 Working with a young person who will not come

A common presentation in adolescent services is the young person who will not attend. Bowenian practice has a distinctive and useful answer to this, and it comes from Petter's work on the family member who refuses to participate.

The refusal is itself a systemic communication, and it usually communicates one of a small number of things: that the young person is protecting the family from a conflict the family cannot tolerate; that the young person has been cast as the family's problem-solver and the refusal is a refusal of that role; that the young person is in a position in which the family is negotiating through them and the negotiation is unwinnable; or that the family system will not tolerate the young person's presence.

The response is not to insist on the young person's attendance. It is to work with whoever will come, to treat the refusal as a piece of clinical information about the family, and to establish a direct line of communication with the young person that does not pass through the parents.

## 9. Case Illustration: The Young Woman Who Had Become Her Mother's Confidant

**Presentation.** Ruth is seventeen and lives with her mother, her younger brother, and her mother's partner. She is referred by the school because of what the school calls "anxiety and possible eating problems". Her mother, who attended the first appointment, presents a picture of a young woman who is doing too much, who is anxious, who has stopped eating properly, and who is "very responsible".

**The individual session.** Ruth's own account is different in every particular from her mother's. She describes a household in which she is the person who holds everything together: she does the shopping, she manages her mother's diary, she cares for her brother after school, she mediates between her mother and her mother's partner, and she has been the principal confidant of her mother's difficulties since her father left when Ruth was eleven. She describes her own account of the eating as "not really about food" and, when pressed, as being about the fact that she is the only person in the house who anybody talks to, and that she cannot go anywhere or do anything without someone needing her.

She says one thing that changes the case: "If I stopped doing all of it, I don't know what would happen. I think it would all land on my mum."

**The genogram.** The genogram is drawn in the second session, with Ruth. It shows: a maternal grandmother who had been treated for a "nervous breakdown" after the birth of her second child; a mother who was the eldest of two and who had assumed responsibility for her mother's care from the age of eleven; a father who left when Ruth was eleven; a stepfather who is present in the household but who is not, in Ruth's account, available to her; and a family pattern in which responsibility passes to the eldest daughter. The emotional level of the genogram shows the alliances: Ruth and her mother, and a complete absence of an alliance between Ruth and her mother's partner, whose participation in the family is limited and who is excluded from the confidences.

The pattern is immediately visible, and Ruth sees it. What she says on seeing it is: "Oh. That's my mum and me. That's my mum and her mum, and me."

**The formulation.** Ruth's mother's own history is the same as Ruth's, and Ruth has reproduced it. The family projection process is operating directly: her mother's own emotional needs, and her mother's own unfinished responsibility for her own mother, have been projected onto Ruth, and Ruth has introjected them as her own. The symptoms — the anxiety, the eating, the over-responsibility, the inability to go out — are the consequences of a young woman who is functioning as the emotional and practical parent of a family of three.

The differentiation of the family is very low, and the family is running on the intensity of the Ruth–mother dyad. The triangle is: Ruth, her mother, and her mother's partner, with the third vertex carrying all of the anxiety and all of the blame. There is no other triangle in this family, and the two adults have no relationship to each other that is not routed through the daughter.

**The intervention.** Four elements, and the first is the one that makes the rest possible.

*Element one: the individual work.* Ruth is seen individually throughout, and the work is about the differentiation. The specific interventions are small and concrete. Ruth practises declining a request: the mother asks her to do the shopping and Ruth says, "I can't today, I've got my coursework." The practitioner explores with her what happened when she was a child and her mother was ill, and she recognises, with some difficulty, that the fear of what would happen if she stopped is the inheritance and not a fact.

*Element two: the parental subsystem.* The work with the parents is the essential work, and it is done without Ruth in the room. The first session with the two adults establishes that they have not had a conversation without Ruth present since Ruth was eleven. The second session does something quite specific: it asks the

two adults to talk to each other, directly, for ten minutes, about a topic that is not Ruth, and the practitioner observes what happens. It is not a comfortable exercise, and it is the beginning of the differentiation of the parental subsystem.

*Element three: the genogram with the whole family.* The genogram is presented in a session with all four, and the discussion is about the pattern rather than about Ruth. This is a key technical decision: the family is asked to talk about the pattern in the grandmother and the great-grandmother, and not about Ruth. What the family finds, when they look at the pattern rather than at the symptom, is that there are two women in the family who did this and one who is about to, and that the pattern can stop with Ruth.

*Element four: the practical arrangements.* Ruth's mother and her partner are asked to take on the practical tasks that Ruth is carrying. It is a difficult conversation, and the resistance is predictable: the mother says she does not know how to cook, and the stepfather says he does not know how to manage her. The practitioner treats both statements as material rather than as excuses, and the working arrangement is small and specific — two evenings a week on which the two adults cook, and a specific task that the stepfather takes over.

**The outcome.** At three months, Ruth is eating regularly, is attending school, and is doing less at home. The relationship with her mother's partner has improved, not because of any work directed at it, but because he is now cooking and she is now sometimes at the table. The relationship with her mother is closer and less suffocating, which is what the just relationship looks like in practice.

Ruth's anxiety has not disappeared, and it should not be expected to at seventeen months; the pattern is thirty years deep and the work continues. Her own account at the end is the best available outcome measure: "I can do something on a Tuesday now and nobody falls over."

**The interpretation.** The case illustrates the central claims of the theory. The symptom was not in the young woman. The family projection process was operating across two generations and was visible on a single page of a genogram. The parents' subsystem, which had no channel of its own, was the location of the work. And the outcome measure that mattered — that nobody falls over — is a measure of the family system's capacity to hold itself together, which is what the theory says the work is for.

# 10. Evidence and Research Support

## 10.1 The status of the evidence

Bowen's theory has a thin evidence base, and the student should be able to state this accurately rather than defensively.

**There is no substantial controlled outcome research on Bowenian family therapy as a whole.** Bowen did not conduct it, and the field has not filled the gap.

**The family-based treatment literature is stronger than the theory-specific literature**, and it is in this literature that the actual support for family work with adolescents lies. The findings are:

**Family-based treatment of adolescent eating disorders has good support.** The family-based approaches, in which the parents take a central role in restoring the young person's nutrition and the family addresses the patterns that maintain the disorder, produce better outcomes than the individual approaches, and the effect on the parental attitudes is the mechanism. This is the strongest evidence for the general claim that parental involvement matters.

**Family therapy for adolescent substance use has good support**, particularly the family-based and the multisystemic variants.

**Behavioural family therapy for conduct problems has a strong evidence base**, as described in Chapter 33, and it is a family intervention with a specific behavioural mechanism.

**Multisystemic therapy has a strong evidence base** for serious antisocial behaviour in adolescents, and it is a systemic intervention with the same core premise — that the adolescent's behaviour is maintained in multiple systems and that the systems must be changed — even though its theory is different.

**The genogram has no validated psychometric properties**, and it should not be used as an assessment instrument in any context in which psychometric reliability matters.

### 10.2 What the theory contributes that the evidence cannot capture

The theory's most defensible contribution is a **way of organising the clinical thinking** rather than a treatment with a measurable effect. The genogram and the concepts of the triangle, the cut-off, and the family projection process give a practitioner a language for making sense of patterns that would otherwise be experienced as inexplicable and therefore as personal failings.

This contribution is real and it is undervalued by the evidence, because it is a contribution to the *formulation* rather than to the *outcome*, and because the profession's research methods are better at measuring outcomes than at measuring the quality of a formulation. A student should be able to hold both positions: that the theory is under-researched, and that the framework organises thinking in a way that other frameworks do not.

### 10.3 Limitations of the evidence

**The controlled trials that exist are small**, and the methodological quality is variable.

**The outcomes are proximal** — the resolution of the identified symptom and the improvement of the family functioning — and the distal outcomes, the educational trajectory, the adult relationships, the mental health in the third generation, are not measured.

**The samples are selected**, and the families who complete family therapy are not representative of the families who are referred.

**The allegiance effects are substantial** in the family therapy literature, and the family-based eating disorder trials in particular have attracted considerable methodological scrutiny, including questions about the generalisability of the family approach to a range of severity and a range of family structures.

## 11. Strengths and Limitations of Bowenian Family Systems Theory

### 11.1 Strengths

**It locates the problem.** The theory's central contribution is to move the question from "what is wrong with this young person" to "what is happening in this family", and the move is almost always clarifying.

**It accounts for the resistance.** A family that will not change is described by the theory as a family that has found a reason not to change, and the reason is investigated rather than confronted.

**It de-blames the young person.** The theory is one of the few in this book that actively relieves a young person of the responsibility for the family's state, and the relief is itself therapeutic.

**The genogram is a superb instrument** for assessment, for communication with a team, and for making a pattern visible.

**It is developmentally congruent.** A theory whose central concept is the adolescent's central developmental task fits the stage.

**It is compatible with other modalities.** Bowenian concepts can be combined with behavioural, cognitive, and attachment work without contradiction.

### 11.2 Limitations

**It is empirically thin**, and a student should be honest about this when the approach is presented to a family or a commissioner.

**The circularity risk is real**, and the theory's own answer to it — that the family must be understood in the context of the wider culture — has not been operationalised in a way that reliably prevents the mislocation of structural problems in the family.

**It presupposes a family**, and a substantial proportion of the young people in contemporary services do not have one in the form the theory requires.

**It can absolve the family of accountability.** The most serious misuse of the framework is the deployment of the concepts as a sophisticated-sounding way of explaining a young person's behaviour in terms of the family's dysfunction while changing nothing. A genogram that is drawn and then filed is a way of looking busy.

**It is difficult to learn and difficult to supervise.** The theory is largely transmitted through apprenticeship, and the quality of a Bowenian practitioner varies enormously, and the supervision of this work requires a specific competence.

## 12. Cultural and Ethical Considerations

### 12.1 Culture, family, and the definition of the family

The theory's cultural assumptions are extensive and worth making explicit.

**The definition of the family.** The theory assumes a family with two parents, a nuclear household, and continuity across the generations. In many settings the relevant family is a single parent, a grandparent, an extended kin network, a chosen family, a kinship network that spans a migration, a collective, or a community. The theory's concepts can be applied to these, and a practitioner should learn to apply them to the family the young person actually has.

**The cultural patterning of the family emotional process.** In settings in which the family is a collective rather than a nuclear unit, the individual differentiation that the theory values may be experienced as disloyalty. A young woman who holds a boundary against her mother's wishes in a culture where the mother's authority is absolute is not practising an underdeveloped differentiation; she is risking an exclusion. The theory's concepts must be applied with a working knowledge of the cost.

**The intergenerational respect for elders.** The practice of the genogram surfaces the family's history, and in some families the history includes experiences that the family has not discussed and does not wish to discuss, and the drawing of the genogram can be experienced as an accusation against the grandparents.

**The migration and displacement of families.** For a family separated by migration, conflict, or political violence, the genogram may be a document of loss, and the losses may be the actual clinical material.

## 12.2 The sociopolitical critique

The most serious cultural criticism of the family systems tradition is that it locates the family's difficulties within the family. A young person's school refusal that is produced by a curriculum that does not accommodate her disability, or by a neighbourhood in which it is not safe to walk to school, or by a family that is exhausted by two shifts of work, is not a family emotional process, and a genogram will document the exhaustion and will not explain it.

The framework's own answer — the sociocultural emotional process — is the right answer, and it is under-developed. A practitioner who uses it will do better than one who does not, and the practical requirement is that the practitioner asks, of every family system, what is happening to it from outside: what is the housing like, what are the wages, what is the school system, what is the immigration status, what is the health service, what is the police presence on the street.

## 12.3 Ethical considerations

**Confidentiality and the family.** Working with a family raises the confidentiality question in its sharpest form. A young person who discloses something in an individual session may not want it shared with the family, and the system requires that what is said where is stated in advance, at the level of the specific.

**The young person's right to be heard.** The systemic view must not become a way of overriding the young person's account. The young person is a member of the system and not merely a position in it.

**The right to refuse family involvement.** A young person whose family is unsafe may be appropriately excluded from family work, and a practitioner who pursues family involvement in the presence of ongoing risk is doing harm.

**The use of the genogram.** The genogram must be constructed with the family and never presented to a young person as a document prepared about them. It must be stored securely, and it must be shared only with the people the family has agreed.

**Avoiding the appropriation of the theory for blame.** The most common ethical failure in systemic work is the deployment of the concepts in a way that assigns responsibility for a young person's difficulty to a family who are themselves the most damaged by the circumstances. The sociocultural process is the corrective, and the practitioner must be able to state what the family is up against.

**Power and confidentiality in the wider system.** A genogram may contain information about a family that is relevant to a safeguarding concern, and the practitioner must know the local reporting obligations and the limits of confidentiality in relation to them.

# 13. Key Terms

### Key Terms at a Glance

- **Family systems theory** — The tradition, developed from Bowen's work, in which the family is understood as a nested set of systems governed by the same emotional processes at different scales.
- **Differentiation of self** — The degree to which a person can maintain a clear sense of self within an emotional relationship, thinking, feeling, and acting in congruence with their own convictions.
- **Undifferentiation** — The flooding of thought, feeling, and action by the emotional pressure of a relationship, in which the person cannot think clearly, cannot modulate feeling, and acts rigidly or impulsively.
- **Cut-off** — The process of reducing emotional involvement with the family in order to reduce anxiety, at the cost of the emotional connection.
- **Healthy, internal, and external cut-off** — The three forms of cut-off: the psychological separation that permits functioning, the withdrawal of emotional availability within the family, and the severance of the relationship.
- **Two-person system** — The dyadic relationship, which Bowen identified as the primary unit of emotional life.
- **Triangle** — A three-person system in which tension between two people is relieved by the involvement of a third; the site of nearly all emotional conflict.
- **Comfortable triangle** — A triangle in which the third person is in tension with both others.
- **Conflictful triangle** — A triangle in which the third person is in tension with one side and aligned with the other.
- **Just relationship** — A relationship of contact and mutual support maintained at a low level of emotional intensity; a principal clinical target in adolescent work.
- **Intense relationship** — A relationship characterised by fusion, high emotional intensity, and difficulty in maintaining a defined self.
- **Law of the preservation of self** — The principle that the function of the organism is to preserve its own integrity and to function as a whole.
- **Law of the preservation of the family** — The principle that the emotional system of the family is more important than any individual member of it, and the source of the tension with the first law.
- **Nuclear family emotional process** — The transfer of emotional process from one generation to the next through the parental subsystem.
- **Family projection process** — The transmission of the parents' own unresolved emotional material to a child, and its introjection by the child.

### Key Terms at a Glance (continued)

- **Projection** — The transmission of the parents' unresolved issues, attitudes, and emotional needs onto a child.
- **Introjection** — The child's absorption of the projected material into their own emotional structure.
- **Multigenerational transmission process** — The description of how emotional processes are transmitted across generations through the family projection and the emotional cutoff.
- **Sibling position** — The way in which a child's place in the birth order, the age gaps, and the sex composition shape the family emotional process.
- **Genogram** — A family diagram that records the relationships, the emotional process, and the significant events across three or more generations.
- **The genogram levels** — The concentric levels of the genogram: the social, the psychological, and the medical.
- **Genogram construction** — The clinical procedure of drawing the family diagram with the family, over four to five sessions, structure first, then relationships, then emotional process.
- **Sociocultural emotional process** — The way in which the emotional processes of a family are shaped by the wider culture, and by the material and political conditions in which it lives.
- **Family emotional system** — The overall organisation of the emotional processes of the family.
- **Emotional process in the body** — Bowen's observation that family members' bodies synchronise in the presence of emotional tension, making the body a diagnostic instrument.
- **The family member who refuses to participate** — The young person who declines to attend family work, whose refusal is itself a systemic communication with a small number of identifiable meanings.
- **Cut-off in adolescence** — The young person's withdrawal from a parent, understood as a possible protection from an intolerable fusion rather than as a character defect.
- **Three-generation focus** — The Bowenian practice of working with a family across at least three generations, so that the current difficulty is located in a lineage rather than in a person.
- **The genogram as a shared object** — The use of the genogram as a clinical instrument that the family can look at and discuss together, rather than a document prepared about them.
- **Chronic family problem** — A problem that has been transmitted across generations and is maintained by the family's current emotional process.
- **Sequential in time** — The Bowenian view of a family problem as a sequence over time, in which the current presentation is one moment in a process.

**Key Terms at a Glance (continued)**

- **The family projection of sibling position** — The tendency for a child who occupies the position of a parent's own sibling to receive an intensified projection.
- **Intergenerational continuity** — The transmission of emotional patterns across generations, which the theory treats as the primary clinical subject.

## 14. Chapter Summary

Bowen family systems theory is a framework in which the family is the fundamental unit of emotional life and in which the same processes operate at every scale, from the two-person dyad to the culture. Its eight core concepts are the two-person system, the differentiation of self, triangles, nuclear family emotional process, emotional cutoff, the multigenerational transmission process, the family projection process, and sibling position, with the sociocultural emotional process and the family emotional system completing the framework.

Differentiation of self is the central clinical concept, and the chapter developed it carefully: the capacity to think, feel, and act in congruence with one's own convictions within an emotional relationship. The two poles were described, the undifferentiated position and the cut-off position, and the chapter was explicit that neither is the goal and that a differentiated position is characterised by the capacity to be close and to be autonomous at the same time. Differentiation operates intrapersonally, in the distinction between the facts and the interpretations, and interpersonally, in the capacity to hold two views of a child at once.

The governing laws were stated and their tension explained, with the explicit caution that the theory does not counsel self-sacrifice or family loyalty, and that the resolution of the tension is called differentiation. The clinical significance was then derived: a family with a symptomatic young person typically contains at least one undifferentiated member and at least one cut-off member, and the symptom reduces as the family's differentiation increases.

The chapter treated the triangle in detail, because almost all clinical material with a young person is triadic, and because the four clinical consequences of the triangle are the practical heart of the theory: that the symptom stabilises the triangle, that removing a symptom without replacing the structure releases the tension, that every conflict between two people has a third, and that the adolescent's position between two parents is a developmental fact. The just relationship was described as the principal target of the work.

The nuclear family emotional process, the emotional cutoff, and the family projection process were described as the mechanisms of the intergenerational transmission, with the family projection process explaining the over-responsible young person whom the theory identifies as one of its most recognisable presentations. The genogram was presented as Bowen's most widely borrowed clinical instrument, with its three levels, its four-session construction procedure, its three distinct uses, and its three limits.

The work with young people was developed in terms of the developmental fit, the clinical principles, the four-session sequence, and the case of the young person who will not come, with the family member who refuses to participate given the specific and clinically useful answer that Bowenian practice offers.

The case of Ruth demonstrated the whole framework. Her symptoms were located in a family projection process operating across two generations, visible on a single page; the work with the parental subsystem was the essential work; the genogram was used to make the family look at the pattern rather than at the symptom; and the outcome measure that mattered — that nobody falls over — was a measure of the family's capacity to hold itself together.

The evidence was presented honestly: there is no substantial controlled outcome research on Bowenian family therapy as a whole, the family-based treatment literature is stronger than the theory-specific literature, and the theory's most defensible contribution is to the organisation of clinical thinking rather than to measurable outcomes. The strengths and limitations were set out, and the limitations included the empirical thinness, the circularity risk, the presupposition of a family, the risk of absolving the family of accountability, and the difficulty of learning and supervising the work.

The cultural analysis addressed the culturally specific definition of the family, the cultural patterning of the family emotional process, the intergenerational respect for elders, and the migration and displacement of families, and the sociopolitical critique was stated at full strength: a family system that is exhausted by two shifts of work is not a family emotional process, and the sociocultural process is the corrective that the framework under-develops. The ethical section addressed confidentiality in the family system, the young person's right to be heard and to refuse family involvement, the use of the genogram, the appropriation of the theory for blame, and the power implications of a document that contains a family's history.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the clinical observation that produced the family therapy movement.
2. State the eight core concepts of Bowen's family systems theory.
3. Explain what it means that the framework is scale-invariant, and give an example at three levels.
4. State the two governing laws and explain their tension.
5. Define differentiation of self, and distinguish it from independence, autonomy, and emotional detachment.
6. Describe the undifferentiated and the cut-off positions, and explain why neither is the clinical goal.
7. Distinguish intrapersonal from interpersonal differentiation, with an example of each.
8. Explain the clinical significance of the family member who is most involved and the family member who is cut off.
9. Describe Bowen's observation about the body, and state its three practical consequences.
10. Define a triangle and distinguish the comfortable from the conflictful form.
11. State the four clinical consequences of the triangle, and explain the significance of the third.
12. Explain the just relationship and its role as a clinical target.
13. Describe the nuclear family emotional process and what is transmitted in it.
14. Distinguish the three forms of emotional cutoff and the clinical consequence of each.
15. Describe the family projection process and the presentation it produces.
16. Explain the clinical significance of the sibling position propositions, and the caution about them.
17. Describe the genogram, its three levels, and its four-session construction procedure.
18. State the three uses of the genogram and its three limits.
19. Explain the developmental fit of Bowenian thinking with adolescence.
20. State the six clinical principles of Bowenian work with young people.
21. Describe the four-session clinical sequence and the purpose of each session.
22. Explain the answer that the family systems tradition gives to the young person who will not attend.
23. Describe the case of Ruth and state the three features of it that generalise.
24. State the status of the evidence for Bowenian family therapy, and summarise the family-based treatment literature.
25. State the three limitations of the evidence base.
26. State the six strengths and the five limitations of the framework.

**Review and Discussion Questions (continued)**

27. Explain the four cultural assumptions of the framework and the cost of ignoring them.
28. State the sociopolitical critique and the framework's own answer to it.
29. State the six ethical requirements for systemic work with young people.

## 16. Practice Activities

**Practice and Application**

### Activity 30.1 — Drawing a Genogram (120 minutes, individual)

Draw a three-generation genogram of your own family, using the standard symbols. Record the structure, then the relationships, then the emotional process, then identify the triangles, the cut-offs, and the projected material. Then write two paragraphs: one describing the pattern you found, and one describing the one thing you found that you had not previously known about your own family. The second paragraph is the point of the exercise, and students consistently report that it is the exercise that changes how they think about their own professional history.

### Activity 30.2 — The Triangle Analysis (75 minutes, small group)

In groups of three, take a real case from placement and map every triangle in it: who is in conflict with whom, and who is the third. Draw each triangle. Then identify the symptomatic member and mark the position they occupy in each. Finally, answer the question the theory asks: "who is the third?" for three separate conflicts in the case, and check your answers against the family. Most groups find that the answers are surprising and that the family can confirm them immediately.

### Activity 30.3 — The Genogram Conversation (90 minutes, triadic)

One student plays a sixteen-year-old who has been told that the family "has problems" and who is wary of the exercise; one plays the mother, who is relieved to be asked to talk about her own history; one plays the practitioner, who draws the genogram and asks the questions. The practitioner's task includes asking about the grandmother's own childhood, and handling the moment when the mother becomes upset. The exercise teaches two things that reading cannot: that the genogram is experienced as an invasion or as a relief depending on the family, and that the young person is usually the most suspicious participant and the one who gains most.

### Activity 30.4 — The Sociocultural Audit (60 minutes, individual)

Take a case from placement and write the two accounts. The first is the genogram account: the emotional process in the family, the triangles, the transmission. The second is the sociocultural account: the housing, the income, the shifts, the school, the immigration status, the health service, the transport, the policing. Then write the two paragraphs side by side and assess how much of the first account is actually explained by the second. Students who do this exercise regularly find that a substantial proportion of what looked like a family pattern is a consequence of material conditions, and this is the most important lesson in the chapter.

## 17. Annotated Reading Pathways

**For the theory.** Read Bowen (1976) for the clearest statement, and read Bowen (1978) for the clinical applications. Then read Bishop (2012) for the practice, and Shapiro (2018) for the clearest modern exposition.

**For the instrument.** Read Bowen (1968) for the original paper and then McGoldrick and Randy (2015) for the assessment application. The genogram is a skill that is learned by doing, and the readings support the practice rather than substitute for it.

**For the field.** Read Nichols (2013) for the whole of the family therapy field, including the schools that Bowen did not found, and read Goldenberg and Goldenberg (2013) for the broad orientation. Reading the field's diversity is what prevents the mistake of treating Bowenian theory as the family therapy field.

**For the evidence.** Read the family-based treatment literature on adolescent eating disorders and on adolescent substance use, and read the multisystemic therapy evidence. These are the studies that actually support the claim that parental involvement matters, and they are more useful to a practitioner than the theory-specific research.

**For the critique.** Read the critical literature on the family systems tradition and on the social construction of family dysfunction, and read the evidence on structural inequality and adolescent mental health. The combination of the two is the preparation for the sociocultural work that the framework needs and does not supply.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Family Systems Theory: Bowen and the Generational Project** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is differentiation, triangles, nuclear family emotional process, and the genogram as a clinical tool. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Daniel is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Daniel's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Daniel explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is

expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Daniel chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Daniel has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Daniel can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Family Systems Theory: Bowen and the Generational Project

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Family Systems Theory: Bowen and the Generational Project** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Family Systems Theory: Bowen and the Generational Project** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

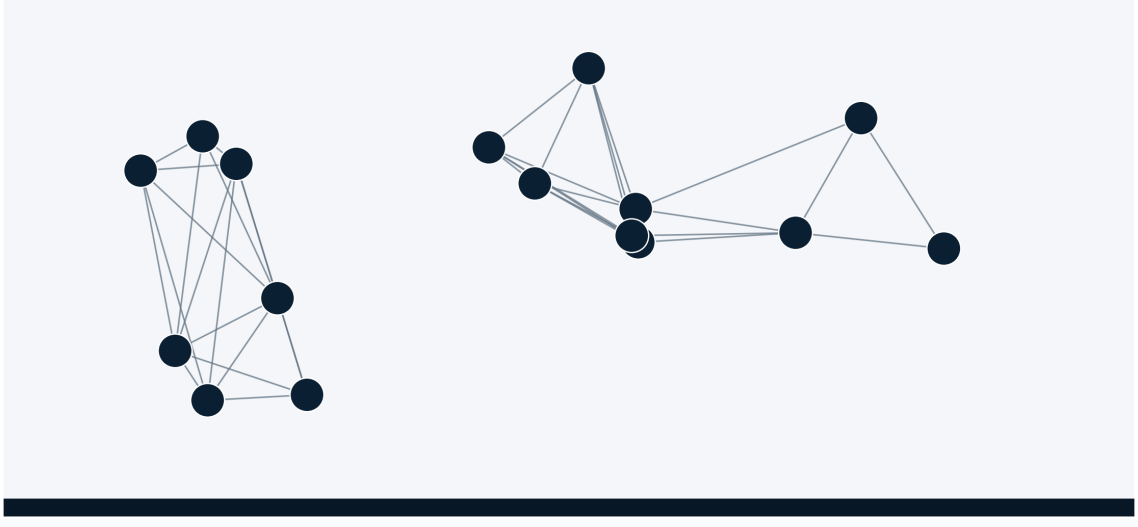
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Family Systems Theory: Bowen and the Generational Project**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Family Systems Theory: Bowen and the Generational Project to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 31

# Structural Family Therapy

Minuchin's structural model, boundaries, subsystems, and restructuring with adolescents and parents.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of the structural model within the family therapy movement and state the clinical problems that produced it.
- Define the five key structural concepts: structure, subsystems, boundaries, hierarchy, and family projection process.
- Explain the concept of the family structure and distinguish it from the family interaction pattern.
- Describe the structure of a family in terms of subsystems, boundaries, and hierarchy, and construct a structural map of a real family.
- Explain the concept of boundaries and their three types, and describe the boundary problems that appear in clinical work with adolescents.
- Describe the principles of structural family therapy, including the use of the structural map, the enactment, the challenge, the restructuring, and the maintenance of change.
- Apply the structural model to a case involving an adolescent and a family, and evaluate the limits of the model.
- Critically evaluate the cultural and ethical dimensions of structural family work with young people.

## Chapter Overview

The structural model was built on a clinical observation that is now so familiar it is easy to miss: **the family is a structure, and the symptoms are structural in origin.**

Salvador Minuchin and his colleagues at the Philadelphia Child Guidance Clinic came to this through work with families of severe, entrenched, and chronic difficulty — families in which the identified patient's symptoms had been present for years, in which previous treatment had not helped, and in which the family appeared to be organised in a way that produced the symptom and resisted any change that did not alter the organisation. The structural therapists' claim was that a symptom in such a family is not an expression of intrapsychic conflict and not a learned behaviour, but a **consequence of the family's structure** — of who is in which subsystem, of where the boundaries are drawn, and of who is in charge of whom.

The model's central intervention is a **restructuring** of the family's organisation, and its central clinical skill is the **enactment**: the invitation to the family to do, in the consulting room, the thing they typically do outside it, so that the practitioner can see the structure in operation rather than hear about it. The enactment was the single most important contribution of the structural school to the technique of family therapy, and it has been adopted, in one form or another, by almost every subsequent school.

The model is a good fit for the work of this book. Adolescents present with structural problems in a way that younger children do not, because the adolescent's developmental task is a reorganisation of the family system, and the structural therapists' concepts of hierarchy, of boundary renegotiation, and of the move from the child subsystem to the adult subsystem are, in effect, descriptions of that task. The problem-solving therapy and the boundary work of the model map directly onto the developmental work of adolescence.

The model's limitations are also worth stating at the outset. The structural model is the least culturally responsive of the family therapy theories, because it assumes a nuclear family, a parental authority structure,

and a set of developmental expectations that are not universal. In Chapter 48, the cultural analysis of the family therapies is taken up in full.

# 1. Historical Context

## 1.1 The Philadelphia Child Guidance Clinic

Minuchin trained as a paediatrician and as a psychiatrist, and his clinical work was at the Philadelphia Child Guidance Clinic, where he led a training programme in family therapy that was the most rigorous in the field for two decades. The clinic's caseload was distinctive: children with severe behavioural and emotional problems, families in which the difficulty had usually been present for years, and families who had been through the child guidance and the individual therapy literature without benefit.

The clinical pressure of that caseload produced the structural theory. If a child's symptom has been present for years, has resisted individual treatment, and is embedded in a family that has developed an elaborate and effective organisation around it, then the organisation is the clinical fact, and the work must be directed at the organisation.

## 1.2 The two problems the model solved

**The problem of the "chronically ill" family.** Minuchin described a family in which a member has been designated as the sick one, and in which the family has organised itself around that designation: the routes of communication are narrowed, the parental roles are reduced, the siblings are conscripted into caretaking or into alignment with the identified patient, and the family has become a stable system whose stability depends on the continuation of the symptom. A treatment that does not restructure the system will produce a change in the symptom followed by its return, because the system has been designed to reproduce it.

**The problem of the "temporarily ill" family.** A second pattern, in which the identified patient is in a position that is transiently untenable — a child who has been assigned a role he cannot sustain, a family in which a parent has been left in charge of both the practical and the emotional management of the children — and in which the symptom will resolve when the structure is corrected.

## 1.3 The relationship to the other schools

The structural model emerged at the same time as the strategic and the Bowenian models, and the three share a premise — that the individual is not the unit of treatment — while differing in what they take to be the primary thing to change. The structural school changes the **organisation**. The strategic school changes the **sequence of interactions and the beliefs about the problem**. The Bowenian school changes the **emotional process and the differentiation**.

The distinctions are worth keeping clear because each generates a different clinical conversation. A structural conversation is about who talks to whom, who decides, and who belongs with whom. A strategic conversation is about the sequence in which things happen and about the attribution of responsibility. A Bowenian conversation is about intensity, differentiation, and the multigenerational pattern.

## 1.4 The development of the model

Minuchin's original formulation was developed with, and refined alongside, his colleagues — particularly Berry, Nagel, and others at the Philadelphia clinic — and it was given its definitive statement in Minuchin's 1974 book *Families and Family Therapy* and its structural-developmental statement in Minuchin's 1978 paper on the structural family treatment of the delinquent adolescent.

Two later developments matter for this book. The first is the **integration of the structural model with the attachment and developmental research** of the 1980s and 1990s, which supplied the developmental criteria for the tasks of the subsystems and made the model more usable with young people. The second is the **development of the structural model with families in poverty and in structurally disadvantaged circumstances**, which is the direction in which the model's assumptions have been most challenged and most usefully revised.

## 2. The Five Key Concepts

### 2.1 Structure

**Family structure** is the organisation of the family, characterised by two features.

**The structure is defined by the subsystems, the boundaries, and the hierarchy**, which are described below.

**The structure is defined by the family's pattern of interaction**, which is the observable and reportable behaviour of the family members: who talks to whom, who is included in what, who defers to whom, who initiates and who responds.

The distinction between the structure and the interaction pattern is important and is a common source of confusion. The interaction pattern is what the family does; the structure is the organisation that the pattern reflects and maintains. A family in which the father speaks to the mother and never to the daughter has an interaction pattern; the structure is the arrangement in which the father is the sole conduit of information and authority to the daughter, and in which the daughter is a member of the child subsystem that is functionally excluded from the adult subsystem.

### 2.2 Subsystems

A **subsystem** is a group of two or more people in the family who are more closely involved with each other than with the others, and which therefore develops its own rules, its own functions, and its own boundaries. Structural therapists have conventionally identified three in the nuclear family, and the terminology is worth using carefully.

**The individual subsystem** is the person.

**The sibling subsystem** is the group of siblings, and it is the unit in which the peer relations of the family are organised. In families with adolescents, the sibling subsystem is often the site of the most active boundary negotiation, and it may be the alliance in which the adolescent's separation is first attempted.

**The parent subsystem** is the marital or spousal unit, and in a family with adolescents this is the unit whose relationship with the child subsystem determines most of what happens.

Two subsystems may be **discrete** — clearly bounded from each other, with no crossover — or **nurturing** — clearly bounded in some respects but with permeability in others, which allows the flow of affection, support, and information while preserving the distinctness of the units.

## 2.3 Boundaries

**Boundaries** are the invisible lines of influence and responsibility that separate the subsystems and the individuals within them. They are the structural model's most important clinical concept, and the one that maps most directly onto the clinical problems of adolescence.

Three kinds of boundary appear in the clinical description.

**Diffuse boundaries** are indistinct, permeable, and non-defining. A person with a diffuse boundary has difficulty differentiating what belongs to them from what belongs to others; the other person's thoughts, feelings, and actions are experienced as their own; and the person is over-involved in others' lives and is unable to act independently. In the family context, diffuse boundaries appear as a parent who cannot allow a child to have an experience that is not shared with her, or a young person who cannot say what he thinks without checking what his mother thinks.

**Clear boundaries** are defined, appropriate to the developmental stage, and permit the flow of affection and support while maintaining the distinctness of the subsystems. A clear boundary between a parent and an adolescent allows the parent to care for the adolescent and to be involved in his life while also allowing him to be his own person, to have private thoughts, and to have relationships the parent does not enter.

**Rigid boundaries** are impenetrable and restrictive, and are typically the result of a history of premature autonomy, of trauma, or of a family in which closeness itself has been experienced as dangerous. A rigid boundary between a parent and an adolescent produces a young person who is emotionally isolated, self-sufficient, and unable to ask for help, and the young person's difficulty in asking for help is frequently mistaken for strength.

## 2.4 Hierarchy

**Hierarchy** is the clear line of authority across the subsystems. The structural model's position is that the parents are in charge of the children, and that the children are not in charge of the parents, and that a family in which the hierarchy is inverted produces symptoms in the children.

The three forms of hierarchy disorder are worth naming.

**Parentification** occurs when a child takes a parental function, either by taking over a practical role — the cooking, the shopping, the care of the siblings, the management of the household — or by taking over the emotional role, by becoming the confidant and the comforter of a parent. Parentification is the structural description of the "over-responsible child", and it is the single most common structural problem in the clinical literature on adolescents.

**Parentification may also run the other way**, in which the parent becomes the child — the parent who confides in the adolescent, who asks the adolescent for advice, who relies on the adolescent for emotional support, or who becomes so focused on the adolescent's needs that the adolescent becomes the caretaker of the parent's emotional life. Minuchin described this as the parent and the child becoming "inverted" in the hierarchy.

**Sibling inversion** occurs when a sibling takes a parental function over another sibling, and it is very common in families with a disabled or distressed child, and it creates a bond that is difficult to dissolve later.

## 2.5 The family projection process

The structural model adopted the family projection process from Bowen, and it uses it in a specifically structural way: the projection of the parents' own difficulties onto a child, and the child's structural role in the family being shaped by that projection.

The clinical significance in the structural frame is that the projected child is usually also the structurally positioned child. A mother who is unable to tolerate her own anxiety projects it onto a child who is placed in a role that requires her to manage it, and the child's symptoms are the medium of the projection.

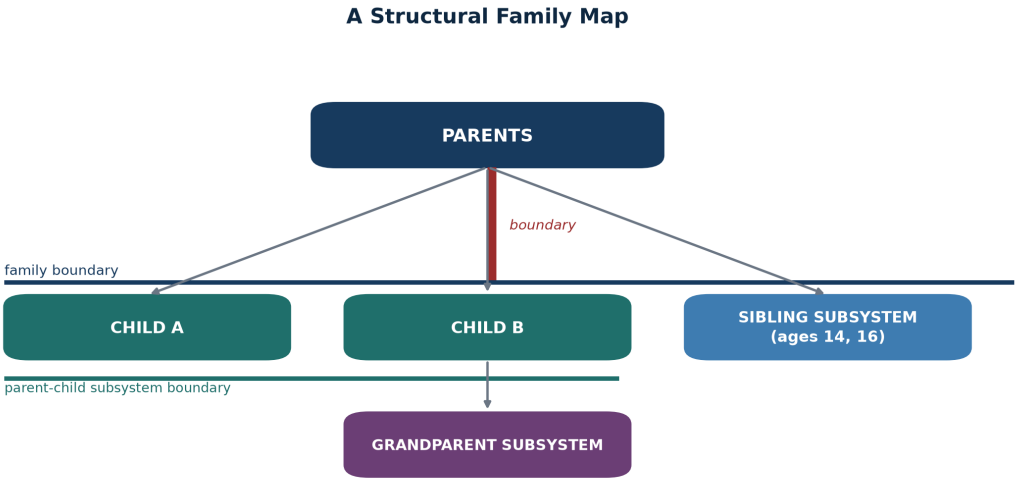



**Figure 31.21** A structural family map showing the parent subsystem, the child subsystem, the sibling subsystem, and the extended family, with the family boundary, the parent-child subsystem boundary, and the generational hierarchy marked.

# 3. Structural Diagnosis

## 3.1 The procedure

The structural diagnosis proceeds in four stages, and it is done with the family present.

**Stage 1: the presenting problem.** The referral question is recorded, and the practitioner asks the family to describe the problem in their own terms. The question that is asked is not "why does she do that?" but "when does it happen, and what happens just before, and what happens just after?"

**Stage 2: the interaction pattern.** The family is asked to describe a recent ordinary occasion in detail, and the practitioner draws the sequence of who did what to whom. This is the "sequence" work, and it is the technique the strategic school also uses; the structural school asks the same question and uses the answer differently.

**Stage 3: the structural map.** The family is drawn in the form of the standard structural diagram, with squares for males and circles for females, with the subsystems delineated, and with the boundaries marked. The drawing is done in the session, with the family's corrections, and the corrections are as informative as the drawing.

**Stage 4: the diagnosis.** The practitioner names the structure: who is in which subsystem, where the boundaries are diffuse, clear, or rigid, where the hierarchy is inverted, and what the family projection process is doing.

### 3.2 The structural map as a clinical tool

The map is used in three ways, and the student should be able to distinguish them.

**As a diagnostic instrument.** The map makes visible the structure that the family cannot see, and the moment of seeing is often a moment of considerable emotional impact.

**As a communication device.** The map can be handed to a school, to a doctor, and to a new practitioner, and it communicates the structure in a form that a non-specialist can use. It is the most portable document in the family therapy field.

**As a therapeutic instrument.** The map is drawn with the family, and the drawing is itself the beginning of the change, because a family that has drawn its own structure has begun to see it.

### 3.3 The distinction between the structural and the interactional formulation

A student should be able to produce both formulations for the same case, because they are different products and they point to different interventions.

The **interactional formulation** describes the sequence: who does what, in what order, with what consequence. It is the basis of the strategic work in Chapter 32 and of the contingency management in Chapter 28.

The **structural formulation** describes the organisation: who is in which subsystem, where the boundaries are, and where the hierarchy is inverted. It is the basis of the structural work.

For the same case, the interactional formulation might be: "the mother withdraws when the daughter asks for anything, and the daughter escalates until she is answered." The structural formulation might be: "the mother and the daughter are in a diffuse boundary, with the daughter functioning as the mother's confidant and the mother functioning as the daughter's emotional regulator; the parental subsystem has collapsed because the father is absent and the grandmother has assumed the parental function with a rigid boundary towards the father."

The structural formulation is the one that predicts what will happen if the practitioner works only on the interaction.

## 4. The Principles of Structural Family Therapy

### 4.1 The first principle: the structure causes the symptom

The first principle is the organising claim of the model: the symptom is a consequence of the family structure, and the symptom will persist as long as the structure persists and will resolve when the structure changes. The clinical implication is that a treatment directed at the symptom alone will fail, and the treatment must be directed at the structure.

## 4.2 The second principle: the family is the unit of treatment

The family is the unit because the structure is the thing that must change, and the structure is not located in any individual. The structural therapists are not neutral about this, and the practice of seeing the family together is not a convenience: a practitioner who sees only the identified patient will be inducted into the family structure as it is, and will be positioned in it, and will be unable to work with it.

## 4.3 The third principle: the intervention is the restructuring

The aim of the intervention is to change the structure: to move a young person from the child subsystem to a position in which they are treated as an adult where the developmental stage requires it, to redraw the boundaries, to restore the hierarchy, and to alter the alliances.

## 4.4 Enactment

The **enactment** is the signature technique of the model and one of the great technical contributions of the family therapy field. It is the invitation to the family to perform, in the consulting room, the interaction that they typically perform outside it, so that the practitioner can observe the structure directly.

**The procedure.** The practitioner asks a family member to describe a recent instance of the difficulty, selects a segment of it, and asks the participants to "go through it" — to enact the exchange as it happened, with the roles they normally occupy and with the words they normally use. The practitioner observes.

**What the enactment reveals.** The practitioner sees the sequence, the alliances, the covert rules, the double binds, the invitations and the refusals. What the family can describe and what the family can *do* are different, and the difference is usually the clinical material.

**The interruption.** After a short time — often less than a minute — the practitioner interrupts, and the interruption is the intervention. The practitioner does not comment on the content; the intervention is behavioural and structural. The classic interventions are: the practitioner asks the young person to do what they have been unable to do; the practitioner asks the parent to stop doing what they have been doing; the practitioner comments on what the family has just demonstrated; or the practitioner enacts a small piece of the interaction themselves in order to make the pattern visible.

**The risk.** The enactment is a challenge, and it can be experienced as a humiliation, particularly if the practitioner interrupts at the moment of the young person's greatest distress, or in a way that sides with the parent against the young person. The structural therapists were not always careful about this, and the modern practice is to time the intervention with care and to attend to the young person's subsequent state.

## 4.5 Challenge and restructuring

The **challenge** is the general term for the interventions that the practitioner makes to change the family's organisation, and the structural therapists' challenge is characteristically direct: naming the process, naming the double bind, and refusing the invitation to be triangulated.

Some of the specific challenges are worth naming.

**The refusal of the triangulation.** When the young person is enlisted as the parent's ally or as the parent's rescuer, the practitioner declines the role and says so. This is the intervention that addresses parentification directly, and it is often experienced by the parent as a betrayal, and it frequently produces an immediate deterioration in the therapeutic relationship with the parent that is temporary and is the sign that the

challenge has landed.

**The naming of the double bind.** A double bind is a situation in which a person receives two incompatible messages, and the restriction on the communication of the message is itself part of the bind. A parent who says "you are an adult now, you make your own decisions" while also saying "if you don't do what I say, you don't respect me" has constructed a double bind, and a young person placed in a double bind is placed in a position from which any response is a symptom. The intervention is to name the bind to the parents, not to the young person.

**The boundary renegotiation.** The most important restructuring with an adolescent is the movement of the boundary between the parent-child subsystem and the adult subsystem. In most clinical work with adolescents, the parents are maintaining a pre-adolescent structure over an adolescent child, and the symptom is a report of the mismatch.

### 4.6 The maintenance of change

The structural therapists were unusually aware that the change achieved in a session decays, and the model has a good account of why: the change is a behavioural change in the family's organisation, and the old organisation is stable because it is familiar, because it is costly to change, and because the other members of the family have not been helped to accommodate the change. A young person who has been given more autonomy and a parent who has not adjusted to the new structure will find the change reversed within a fortnight, and the practitioner should expect it and plan for it.

Three technical requirements follow. The change must be **consensual** — every member of the family must have some stake in it, and a restructuring imposed from outside will be dismantled. It must be **explicit** — the new rules are stated aloud, in the session, and by the family, so that the new structure is a shared project rather than a secret. And it must be **maintained by the family's own mechanisms**, which in practice means the practitioner works with the parent on the parent's difficulties with the new structure, and the parent's difficulties are the material.

## 5. Further Reading for This Chapter

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# 6. Structural Family Therapy with Children and Adolescents

## 6.1 The developmental fit

The structural model is unusually well matched to the developmental stage at which adolescents present, for four reasons that a student should be able to state.

**Adolescence is a structural task.** The adolescent's work is a reorganisation: the movement from the child subsystem to a position of increasing autonomy, the renegotiation of the parent-child boundary, the acquisition of a peer world that the parents do not control, and the eventual establishment of a life that is theirs. These are exactly the structural variables the model names.

**The mismatch is the pathology.** A family that maintains a pre-adolescent structure over an adolescent child produces a symptom. The symptom is the report of the mismatch, and it is a report in the language of behaviour because the young person does not have another channel.

**The parentification problem is developmentally salient.** A young person who is managing a parent's emotional life is being asked to do a developmental task in reverse, and the structural model has the clearest available language for it.

**The intervention is structural.** The restructuring of the boundary and of the hierarchy is precisely the intervention that the developmental task requires, and the model provides the technical means to do it.

## 6.2 The four structural tasks of adolescence

The structural-developmental formulation identifies four developmental tasks, and the assessment of an adolescent is organised around them. A student should be able to use them as a checklist and, more importantly, should be able to recognise a task that has been left incomplete.

**The first task: to differentiate from the family of origin while maintaining a relationship with it.** The failure of this task presents as either fusion — the young person cannot be apart — or cut-off — the young person cannot be together. A family that has not made room for the differentiation will find that the adolescent leaves entirely, or that the adolescent stays and cannot develop.

**The second task: to move from the child subsystem to a more adult position in the family.** This involves the renegotiation of the boundary, the assumption of adult responsibilities, and the participation in adult decision-making. The failure of this task presents as a young person who is treated as a child in the

family's structure and who responds with the only available means.

**The third task: to develop peer relationships of increasing depth and mutuality.** The family structure must permit the peer world, and the parents' difficulty in permitting it is usually a difficulty with their own fear.

**The fourth task: to develop a vocational and financial direction that enables leaving home.** This is a task that is often overlooked in clinical work with adolescents and it is a common source of family conflict that is misread as a behavioural problem.

## 6.3 Boundary problems in clinical practice

Four boundary presentations recur, and each requires a different intervention.

**The diffuse boundary.** The parent and the adolescent are so close that the parent's feelings are the adolescent's feelings and the adolescent's are the parent's. The clinical presentations are: the adolescent who cannot make a decision without checking with the parent; the parent who cannot tolerate the adolescent's distress and must resolve it immediately; and the adolescent who reports a sense of being responsible for the parent's emotional state. The intervention is the establishment of a clear boundary, which is painful and which is usually resisted by the parent, because the parent experiences the boundary as a loss.

**The rigid boundary.** The parent and the adolescent are separated, and the separation is usually the residue of something: an earlier closeness that ended in conflict, a loss, a previous family breakdown, a history of the parent being hurt by the same behaviour. The clinical presentations are: the adolescent who is emotionally unavailable, who does not share anything, who has no visible distress and who cannot be reached; and the parent who says the adolescent is "fine" and is puzzled by the referral. The intervention is a gradual increase of contact, and the risk is that the increase of contact re-ignites whatever produced the rigidity.

**The two-boundary problem.** The adolescent is closely enmeshed with one parent and rigidly distant from the other. This is the classic triangulated adolescent presentation, and it is very common where a family has separated or where one parent is absent. The intervention is directed at the parental subsystem and not at the adolescent, and a practitioner who works with the adolescent alone will achieve very little.

**The boundary that oscillates.** The family alternates between intense closeness and complete distance, and the adolescent learns that the distance follows the closeness. The oscillation is usually maintained by the parent's own unresolved process, and the intervention is the work with the parent, and the adolescent's stability improves when the oscillation stops.

## 6.4 The clinical sequence

**Session 1: the individual.** An assessment with the young person, alone. The purpose is to hear the young person's own account of the family and to establish a relationship that is not mediated by the parents. This session is a structural intervention in itself, because it establishes the young person as a person with a perspective that is not the family's.

**Session 2: the family, and the map.** The family session in which the structure is drawn. The practitioner asks the family to describe a recent instance of the difficulty and to enact it. The drawing of the map follows.

**Session 3: the challenge.** The intervention session, in which the practitioner names the structure, refuses the triangulation, and makes the restructuring explicit. These sessions are frequently uncomfortable and frequently produce a temporary deterioration in the relationship with one or both parents, and the practitioner should be prepared for it and should have said in advance what will happen.

**Session 4 onwards: the maintenance.** The work with the parents on the new structure. The young person is seen individually and the family is seen together, and the balance depends on the case.

### 6.5 Work with the parents when the young person will not attend

A common situation is an adolescent who refuses to attend family sessions. The structural model has a clear answer, and it is more pragmatic than the Bowenian one.

The practitioner works with the parents alone, and the adolescent's absence is treated as a piece of structural information rather than as an obstacle. The parents are asked what the adolescent says about coming, and the answer is usually a statement about the family rather than about the therapy: "she says you'll take her side", "he says we'll all just criticise him", "she says you always talk about her when she's not there." Each of these is a description of a structural problem, and each of them can be worked with.

The practitioner also keeps a direct channel open with the young person, and the most useful practical step is often to shift the individual work to a different setting — a school, a text message, a walk — so that the young person is not in a position in which attendance means entering a structure they have named as unsafe.

## 7. Case Illustration: The Young Man Who Was Not Allowed to Grow Up

**Presentation.** Declan is sixteen and lives with his mother, his father, and a younger sister of eleven. He is referred by the school for "oppositional behaviour and possible ADHD". His mother attends the first appointment and describes a young man who is argumentative, who does not do his chores, who does not do his homework, who is in his room with his door shut, and who is "not the son I raised".

**The individual session.** Declan's own account is different. He describes a household in which he is the family's electrician, the family's driver, the family's shopper, the person who deals with the boiler, the person who looks after his sister when his mother works, and the person who is told to be "a man" by his father and not to be "a baby" by his mother. He says his mother rings him from work when the heating breaks. He says his father has never once asked him about school. He says that if he wants to go to a friend's house he has to have a reason and a time and a plan for getting back.

He also says something that reframes the referral: "You know what the problem is? There's no room for me to be fifteen. I'm either a little kid or I'm a man. There's nothing in between."

**The family session and the map.** The family is invited in and the practitioner asks each of them to describe a recent instance of the difficulty, and the family cannot agree. Declan's father describes an incident last week in which Declan did not come down for dinner. His mother describes an incident in which Declan did not answer the phone when she called from work. Declan describes a different incident entirely, in which his father told him he would be "kicked out" for coming home late.

The practitioner then asks the family to enact the most recent of these, and the mother begins to describe the phone call, and Declan's voice immediately changes, and he says, in a flat and sarcastic register, "I can't do anything right, can I?" — and the practitioner observes the father's immediate shift from the sofa to

standing, and the mother's hand going to her mouth, and the whole family reorganising around a fifteen-year-old's sarcastic sentence.

The practitioner interrupts at that point and says: "What I see is that Declan said one sentence and the three of you changed. Declan, what did you want to happen when you said that?" Declan says: "I wanted him to sit down." The practitioner asks his father whether he sat down. He did not. Declan says: "That's what I mean."

**The structural map.** The map is drawn in the session, and it looks like this. There is a parental subsystem in which the mother and the father are joined by a boundary that is diffuse at the point of contact with Declan and rigid at the point of contact with each other. There is a child subsystem of Declan and his sister, in which the siblings are close. The critical feature is between the parents and Declan: the boundary is *clear* in the direction of responsibility and *diffuse* in the direction of emotional involvement, and the map shows this as a line that is solid on one side and dotted on the other.

The projection process is also visible. Declan's mother is a single parent of a nine-year-old sibling when Declan was born, and the grandmother was the primary carer for the first three years. The grandmother died when Declan was four, and the mother describes having been "in pieces" for a year. The father met the mother when Declan was five and has never, by his own account, known how to talk to him; he says "I've never been one of those dads who does the talking." The projection of the grandmother's caretaking onto the mother, and the mother's fear of the loss of another person, are being carried into Declan's role as the family's competent one.

**The formulation.** Declan is not oppositional. He is *parentified*, and the parental subsystem is *collapsed in its parental function*, with the grandmother's function having been taken over by Declan. The family's difficulty is that the adult roles were never re-established after the grandmother's death, and the structure has been maintained for twelve years by the most efficient possible arrangement: the oldest child does it.

The symptom — the argumentativeness, the sarcasm, the closed door, the refusal to do homework — is the report of a young person who is a functional adult in a family that will not let him be fifteen.

**The intervention.** Four elements, and the sequencing matters.

*Element one: the parental subsystem.* The practitioner works with the parents alone for the first four sessions, and the work is not about Declan at all. The first session establishes that they have not spoken to each other about anything other than the children and the bills since Declan was five. The second session addresses the father's account of his own history with his own father, which is a short, angry, and revealing account, and the third addresses the mother's, which is a long account of a loss and of a household that held itself together. The fourth session is a joint session in which the practitioner asks them to describe each other's difficulties, and asks them to say it to each other rather than to him. It is a difficult and unglamorous session and it is the essential one.

*Element two: the boundary renegotiation.* The restructuring is specific and it is written down. Declan will have his own room and his own door, which will be closed when he wishes, and the family will not comment on it. Declan will have one evening a week when he is responsible for a specified household task, and the other two evenings his mother and his sister will manage without him, and this is the hardest part of the plan for the mother. Declan will not be the emergency contact for the heating, the shopping, or the school, and the practitioner will say this to the family explicitly and will then say why: "You have been running this house with a twelve-year-old in a man's job and it has nearly worked. It cannot go on, and it is not fair to him, and I am not going to let the household fail when it happens, so we are going to plan for it."

The practitioner says the last part in the family session, in front of Declan, because the unspoken fear is that the family will collapse when Declan stops, and saying it aloud is what makes the restructuring possible.

*Element three: the father's role.* The father's role is specified and it is small. One specific task, done on a specific day: he will drive Declan and his sister to the football match on Sunday, and they will go to the cafe afterwards, and this is a routine, and routines do not require intimacy. The practitioner explains to the father that Declan does not want a relationship that is emotionally intense; he wants a relationship that is predictable and does not require him to perform.

*Element four: the individual work with Declan.* Declan is seen throughout. The work is about the anger, which is enormous and is entirely justified, and about the difference between the anger at his family and the sarcasm, which is a defence that costs him the room he is in.

**The outcome.** At four months, Declan is still argumentative and is markedly less sarcastic. The chores are done, mostly, and the household tasks have been transferred with two significant relapses, both of which occur when his mother is under pressure at work and both of which are workable. The father and the son have a Sunday routine and have had three arguments in it, which the father reports as "progress, she says". The mother reports that she is "exhausted but different".

**The interpretation.** The case illustrates the structural model almost completely. The symptom was structural in origin. The intervention was a restructuring of the boundary, of the hierarchy, and of the parental subsystem, and the individual work with Declan was necessary and was not sufficient. The single most important moment in the whole case was the practitioner's refusal to allow the family to collapse silently, and the naming of the fear in front of the young person. A practitioner who had worked only on Declan's sarcasm, or only on his mother's anxiety, or only on the father-daughter relationship would have achieved a fraction of what was achieved.

# 8. Evidence and Research Support

## 8.1 The status of the evidence

The structural model has a modest evidence base, and the specific findings are as follows.

**There is no strong controlled evidence for structural family therapy as a whole.** The model's own tradition is more clinical than experimental, and the outcome literature on the specific model is limited.

**The evidence for family-based treatment of adolescent eating disorders is strong**, and the family-based approaches in this area share the structural premise that the parents must be restored to a central and authoritative position in relation to the young person's eating and that the parentification of the mother by the daughter must be interrupted. The family-based approach to anorexia nervosa and to bulimia nervosa is one of the clearest demonstrations in the field that a structural change in the family produces a change in the symptom.

**The evidence for behavioural family therapy for conduct problems is strong**, and the behavioural family therapy protocols are the most thoroughly validated family interventions for oppositional and conduct disorder. They are not structural family therapy, but they are family interventions with a clear structure: the parents are the agents of change, the child is the target, and the parent-child interaction is the medium.

**The evidence for multisystemic therapy is strong**, and the model is a systemic intervention with the same core premise.

**There is modest evidence for structural family therapy with child and adolescent conduct problems**, from a small number of controlled trials, and the results are more variable than those of the behavioural approaches.

### 8.2 What the evidence cannot settle

The question of whether structural family therapy is better than other family approaches is largely unanswerable, because the models are rarely compared and the therapists who practise them are rarely the therapists who evaluate them. The honest position is that the model provides a coherent account and a set of techniques, and that the specific interventions within it have variable support, and that the practitioner should be clear about which of their interventions are evidence-supported and which are technique.

### 8.3 Limitations of the evidence

**Small samples and high allegiance.** The family therapy literature has among the highest allegiance effects in the field, and the structural tradition has been particularly prone to a strong theoretical commitment.

**Proximal outcomes.** The family therapy literature overwhelmingly measures the resolution of the identified symptom, the family functioning, and the satisfaction of the referrer, and rarely measures the distal outcomes.

**The problem of the "response of the identified patient".** In family research, the young person's own account of improvement is often excluded in favour of the parents' account, on the grounds that the parents are more reliable. This is a serious methodological error in a field whose subject is the family, and it has been corrected in the more recent literature.

## 9. Strengths and Limitations of the Structural Model

### 9.1 Strengths

**It identifies a cause that can be changed.** The model's claim that the structure causes the symptom is unusual in psychotherapy, and it is a claim that points directly at something that can be worked on.

**It gives a clear description of the pathology.** A practitioner who can say "the parental subsystem has collapsed and the child is functioning as a parent" has a description that is immediately useful, that the family can understand, and that predicts what will help.

**It has a technical repertoire.** The enactment, the challenge, the map, the boundary renegotiation, and the maintenance of change are specific and teachable.

**It fits the adolescent developmental task**, as described above.

**It is compatible with other modalities**, and the structural work is frequently combined with behavioural, cognitive, and attachment work.

**The map is a portable document** that can be used with a school, with a new practitioner, and in a referral.

### 9.2 Limitations

**It is culturally narrow.** The assumptions about the nuclear family, the parental authority, and the appropriate developmental path are specific to a particular cultural context, and the model's language of hierarchy and of parental responsibility carries implicit value judgements that do not transfer.

**It can be used to enforce conformity.** The model's language of the parent's authority and of the child's place can be deployed in ways that are not therapeutic at all, and the boundary work of Chapter 48 is required to prevent this.

**The challenge can be a confrontation.** The structural therapists' willingness to interrupt an enactment in the moment of the young person's greatest distress was, in retrospect, too confident, and the modern practice is more careful about the timing and the aftermath of the intervention.

**It under-accounts for the individual.** A young person who is depressed, or autistic, or has a trauma history, has difficulties that are not structural, and a model that explains everything structurally can obscure them.

**The evidence is modest**, and the model has been rather poorly served by the research literature.

# 10. Cultural and Ethical Considerations

## 10.1 The cultural assumptions of the model

The structural model embeds a set of assumptions about family life that are not universal, and a student who does not examine them will impose them.

**The nuclear family with two parents.** The model assumes a parental subsystem, and the assumption fails for single-parent families, for families with three or more generations, for separated and never-married parents, for adoptive families, for families with a same-sex couple, and for families in which the kinship system is the extended family or the community.

**The hierarchy.** The model's position that the parents are in charge of the children is a position about parental authority, and parental authority varies enormously across cultures, families, and the contexts in which the young person lives. In some families and communities, the elder generation has authority and the parents do not; in some, the young person is expected to be self-managing from an early age; in some, the family's decisions are made collectively.

**The developmental path.** The structural-developmental formulation assumes a particular sequence: dependence, then differentiation, then autonomy, then leaving. Many young people follow a different sequence, and a family that does not follow the assumed sequence is not thereby pathological.

**The concept of the boundary.** The boundary metaphor is powerful and it is culturally specific. In cultures in which the self is understood as relational, in which interdependence is a value and not a failure of differentiation, the "too close" language of the model is a poor fit, and a family that is described as fused may simply be a family that functions well.

## 10.2 The risk of the model's misuse

The most serious risk in the application of this model is that it becomes an instrument of enforcement. The language of hierarchy, of the parent's authority, and of the child's place is readily turned into a justification for controlling a young person's behaviour, and the model's technical vocabulary lends an authority to that claim that it does not deserve.

Three safeguards follow. The practitioner should be alert to the moment when the work stops being about the young person's development and becomes about the parents' control of the young person. The young person's own account of the structure should be sought directly and weighted appropriately. And the practitioner should maintain the wider frame of Chapter 48, in which the family's structure is examined against the conditions the family lives in.

### 10.3 Ethical considerations

**Consent to family work.** A young person's consent to family sessions is required, and the structure of the sessions should be explained. A young person who is attending under protest is a young person who will be described as resistant, and the description will be believed.

**Confidentiality in a family setting.** Confidentiality cannot be absolute in a family session, and the limits should be stated at the beginning, in terms the young person can act on. The structural therapists' practice of asking the young person to keep the contents of an individual session private from the parents is ethically problematic and should be approached with care.

**The young person's right to a view.** The practitioner should not present a structural formulation to a family as an established fact. A formulation that is presented as a diagnosis will be experienced as a verdict, and the young person will be invited to accept a description of their family that they did not have a part in writing.

**The protection of the young person from the parental subsystem.** Where the work involves a challenge to a parent, the practitioner must anticipate the possibility that the parent will retaliate against the young person, and must plan for it.

**The limits of the work.** A structural intervention is an intervention in a family's organisation, and the practitioner should be clear about the limits of their mandate, about the limits of their competence, and about when the case requires a different or additional response.

**Equity of participation.** The structural model presumes that all family members can participate. The practitioner must consider the family members who cannot — a young person with a communication impairment, a parent with a learning disability, a family member who does not speak the language of the sessions — and must adapt rather than exclude.

## 11. Key Terms

**Key Terms at a Glance**

- **Family structure** — The organisation of the family, defined by its subsystems, its boundaries, and its hierarchy, and reflected in its pattern of interaction.
- **Interaction pattern** — The observable pattern of who does what to whom, as described by the family and as enacted in the consulting room.
- **Subsystem** — A group of two or more family members more closely involved with each other than with the others, with its own rules, functions, and boundaries.
- **Individual subsystem** — The person as the smallest unit of family organisation.
- **Sibling subsystem** — The group of siblings, and the unit in which the family's peer relationships are organised.
- **Parent subsystem** — The marital or spousal unit, whose relationship with the child subsystem determines much of what occurs with adolescents.
- **Discrete subsystem** — A subsystem that is clearly bounded from the others, with no crossover.
- **Nurturing subsystem** — A subsystem that is bounded in some respects and permeable in others, allowing the flow of affection and support while preserving distinctness.
- **Boundary** — The line of influence and responsibility separating the subsystems and the individuals within them; invisible but functionally decisive.
- **Diffuse boundary** — An indistinct and permeable boundary, in which the other person's thoughts, feelings, and actions are experienced as one's own.
- **Clear boundary** — A defined boundary appropriate to the developmental stage, permitting the flow of affection and support while maintaining distinctness.
- **Rigid boundary** — An impenetrable and restrictive boundary, usually the residue of a history in which closeness was experienced as dangerous.
- **Hierarchy** — The line of authority across the subsystems, in which the parents are in charge of the children.
- **Hierarchy reversal** — A parent and child who have exchanged roles, with the parent seeking direction and emotional support from the child.
- **Parentification** — A child who takes a parental function, in the practical or the emotional domain, or both.
- **Sibling inversion** — A sibling who takes a parental function over another sibling.
- **Structural map** — The standard diagram of the family, with the subsystems delineated and the boundaries and the hierarchy marked, drawn with the family in session.

**Key Terms at a Glance (continued)**

- **Enactment** — The invitation to a family to perform in the consulting room the interaction they typically perform outside it, so that the practitioner can observe the structure.
- **The interruption** — The intervention made during an enactment, in which the practitioner changes what the family is doing rather than commenting on what it has said.
- **Challenge** — The general term for the interventions by which the practitioner attempts to change the family's organisation.
- **Restructuring** — The goal of the structural interventions: the movement of a young person from the child subsystem to an adult position where the developmental stage requires it, the redrawing of the boundaries, and the restoration of the hierarchy.
- **Double bind** — Two incompatible messages given to a person with a restriction on the communication of the message, a position from which any response is a symptom.
- **Refusal of the triangulation** — The structural intervention in which the practitioner declines the role the family has assigned them and names the process.
- **Maintenance of change** — The work of keeping a restructuring in place, by securing the family's consensual and explicit commitment and by working with the parent's difficulties with the new structure.
- **Boundary renegotiation** — The developmental task of moving an adolescent from the child subsystem towards an adult position, with the accompanying changes to responsibility and participation.
- **The four structural tasks of adolescence** — Differentiation from the family of origin, the move from the child subsystem, the development of peer relationships, and the acquisition of a vocational and financial direction.
- **Two-boundary problem** — An adolescent who is closely enmeshed with one parent and rigidly distant from the other, the classic triangulated presentation.
- **Oscillating boundary** — A family that alternates between intense closeness and complete distance, so that the distance follows the closeness.
- **Enacting the sequence** — The clinical technique of asking a family member to describe a recent ordinary instance of the difficulty in detail, in order to obtain the interaction pattern.
- **Family projection process in the structural frame** — The projection of the parents' own difficulties onto a child whose structural position is shaped to receive it.

## 12. Chapter Summary

The structural model was built on the claim that the family is a structure and that the symptoms are structural in origin. Minuchin's work with families of severe and entrenched difficulty produced the claim, and the model's central intervention is a restructuring of the family's organisation, with the enactment as its signature technique.

The five key concepts were developed in turn. Structure is defined both by the subsystems, the boundaries, and the hierarchy and by the observable pattern of interaction, and the distinction between the two was emphasised because each generates a different clinical conversation. Subsystems were described with the terms discrete and nurturing. Boundaries were described in their three forms — diffuse, clear, and rigid — with the clinical presentations of each, and with the observation that the rigid boundary is usually the residue of a history in which closeness was dangerous. Hierarchy was described with its three disorders: parentification, hierarchy reversal, and sibling inversion. The family projection process was described in the specific structural sense in which the projected child is also the structurally positioned child.

Structural diagnosis was described as a four-stage procedure, and the distinction between the structural and the interactional formulation was made explicit with a worked example, because the two produce different interventions and the structural formulation is the one that predicts what will happen if the practitioner works only on the interaction.

The five principles of the model were then developed: the structure causes the symptom, the family is the unit of treatment, the intervention is the restructuring, the enactment is the means of diagnosis, and the change must be maintained. The enactment was described in detail — the procedure, what it reveals, the interruption, and the risk — with the explicit warning that the modern practice is more careful than the original about the timing of the interruption and the young person's state afterwards. The maintenance of change was described with the three requirements of consensus, explicitness, and the family's own maintenance mechanisms.

The developmental fit with adolescence was developed through the four structural tasks, and the four boundary problems were described with the intervention appropriate to each. The clinical sequence was given, and the case of the adolescent who will not attend was treated with the model's pragmatic answer, in which the refusal is read as structural information.

The case of Declan demonstrated the model in full. His symptoms were not oppositional but the report of a young person who was parentified in a family whose parental subsystem had collapsed and whose grandmother's function had been absorbed by him. The enactment produced the decisive moment — the family reorganising around a sarcastic sentence — and the practitioner's interruption asked the question that produced the formulation. The intervention was four elements, and the most important of them was the practitioner naming, in front of the young person, the family's unspoken fear that it would collapse when he stopped, and promising that it would not.

The evidence was presented honestly: the model has modest controlled evidence, and the stronger evidence in the adjacent areas is for family-based treatment of eating disorders, for behavioural family therapy, and for multisystemic therapy. The evidence's limits were named, including the serious methodological problem of the exclusion of the identified patient's own account.

The strengths and limitations were set out, and the limitations included the cultural narrowness, the risk of the model being used for enforcement, the over-confident challenge, the under-accounting of the individual, and the modest evidence. The cultural analysis examined the assumptions of the nuclear family, of the hierarchy, of the developmental path, and of the boundary metaphor itself, and the ethical section addressed consent to family work, confidentiality in the family setting, the young person's right to a view, the protection of the young person from the parental subsystem, the limits of the mandate, and the equity of participation.

# 13. Review and Discussion Questions

## Review and Discussion Questions

1. Explain the clinical problem that produced the structural model.
2. State the five key concepts of the structural model.
3. Distinguish the family structure from the interaction pattern, and give an example of each for the same family.
4. Define a subsystem, and explain the terms discrete and nurturing.
5. State the three types of boundary, describe the clinical presentation of each, and specify the intervention for each.
6. Explain hierarchy, and describe the three forms of hierarchy disorder.
7. Describe the family projection process in the structural sense.
8. State the four stages of the structural diagnostic procedure.
9. State the three uses of the structural map.
10. Produce both an interactional and a structural formulation for the same case, and explain how they point to different interventions.
11. State the five principles of structural family therapy.
12. Describe the enactment procedure, and state the four kinds of interruption.
13. Explain the risk of the enactment, and describe the modern caution about it.
14. Describe the three specific challenges: the refusal of the triangulation, the naming of the double bind, and the boundary renegotiation.
15. State the three requirements for the maintenance of change, and explain why each is necessary.
16. State the four reasons why the model fits the adolescent developmental stage.
17. State the four structural tasks of adolescence and the presentation of the failure of each.
18. Describe the two-boundary problem and the oscillating boundary.
19. State the four-session clinical sequence and the purpose of each session.
20. Describe the structural answer to the adolescent who will not attend family sessions.
21. Describe the case of Declan and state the three features of it that generalise.
22. Summarise the status of the evidence for the structural model and for the three adjacent family approaches.
23. State the four cultural assumptions of the model.
24. Explain the risk of the model's misuse as an instrument of enforcement, and the three safeguards.
25. State the six ethical requirements for structural work with young people.

## 14. Practice Activities

**Practice and Application**

### Activity 31.1 — Drawing a Structural Map (90 minutes, field-based)

In a placement, take one family and draw the structural map: the subsystems, the boundaries, and the hierarchy. Mark each boundary as clear, diffuse, or rigid, and justify each judgement from observed or reported interaction. Then write a sentence naming the position of each young person in the family. Do not share the map with the family. The exercise teaches the difference between the structure as a description and the structure as a verdict, and the third stage of the activity — the position of the young people — is where the interpretive leap has to be made.

### Activity 31.2 — The Enactment Script (75 minutes, small group)

In groups of four, one student plays a sixteen-year-old, one plays the mother, one plays the father, and one plays the practitioner. The practitioner asks the family to enact a recent instance of a conflict, and the practitioner's task is to observe and then to interrupt. The three family members then describe what happened from their own positions, and the practitioner describes what they saw. The exercise is the most instructive in the book for the enactment, because the students discover that what the family can describe and what the family can do are quite different things, and that the practitioner's intervention is a change in the behaviour rather than a comment about it.

### Activity 31.3 — The Cultural Audit of a Map (60 minutes, individual)

Take a structural map from a family in a context that is not the one in which you were trained — a different cultural setting, a different family form, a different expectation about the developmental path — and audit it. For every element of the map, ask: is this a description of this family, or is this an expectation imported from my own context? Rewrite the map so that it describes the family as it is, without the imported expectations. Most students find that a substantial part of the original map was expectation, and this is the most useful single lesson in the chapter.

### Activity 31.4 — The Refusal of the Triangulation (60 minutes, pairs, discussion)

In pairs, one student plays a parent who has been confiding in their sixteen-year-old about their marital difficulties, and the other plays the practitioner. The practitioner must refuse the role — decline to be drawn into the confidences, name the process, and explain to the parent why the arrangement is being changed. The second half of the exercise is the harder part: the practitioner must then describe what will happen next, including the fact that the parent's relationship with the practitioner may deteriorate, and including what the practitioner will do about the young person in the meantime. The preparation for this conversation is more useful than almost any other exercise in Part V.

## 15. Annotated Reading Pathways

**For the model.** Read Minuchin (1974) and then Minuchin and colleagues (1981), and read Minuchin and Nichols (1993) for the modern statement. Read the 1978 paper as the shortest complete statement of the model.

**For the technique.** Read Minuchin, Sell, and Montalvo (1998) for the clinical practice, and read the chapters on enactment and on the challenge in any current structural text. The technique is learned by supervision and by practice, and the readings support rather than substitute for it.

**For the field.** Read Nichols (2013) for the whole family therapy field, so that the structural model can be placed alongside the strategic and the Bowenian. Reading only the structural school is how a student ends up unable to compare.

**For the evidence.** Read the family-based treatment literature on adolescent eating disorders, read the behavioural family therapy evidence, and read the multisystemic therapy evidence. These are the studies that actually support the parental-involvement claim, and the structural model's central clinical assertion.

**For the critique.** Read the critique of structural family therapy on the grounds of social construction, and read the critical literature on the use of family therapy in contested and safeguarding contexts. The combination is the preparation for the work in Part VIII.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Structural Family Therapy** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is minuchin's structural model, boundaries, subsystems, and restructuring with adolescents and parents. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning,

language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Nia is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Nia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Nia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Nia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what

will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Nia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Nia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.

- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Structural Family Therapy

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Structural Family Therapy** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Structural Family Therapy** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a

professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

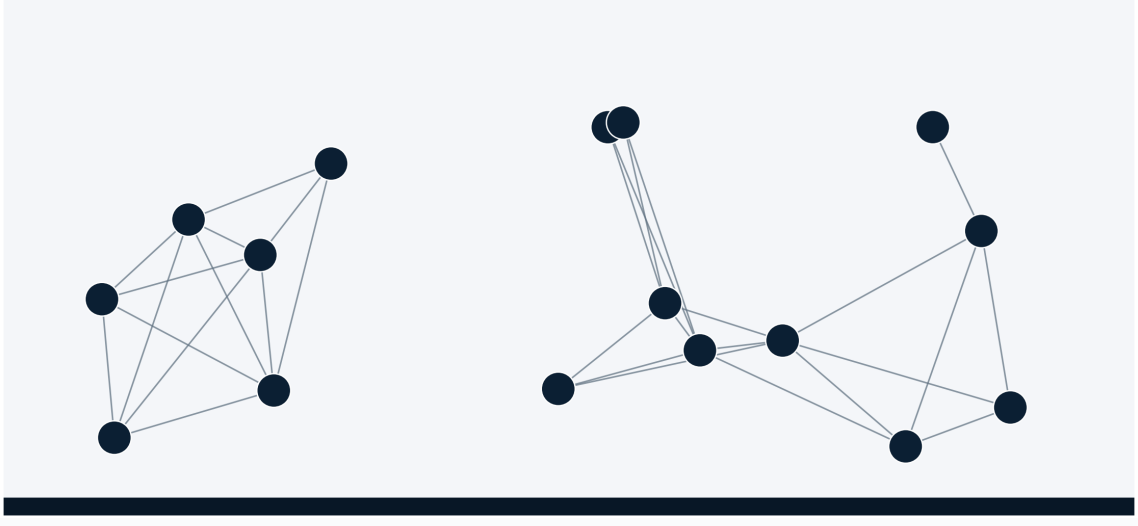
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Structural Family Therapy**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Structural Family Therapy to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 32

# Strategic Family Therapy and Communication Theory

Problem-maintaining sequences, directives, and the reframing of the problem.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of the strategic school and describe the clinical problems that produced it.
- State the core concepts of the strategic model, including the problem-maintaining sequence, the double bind, the symptom as a metaphor, and the reframing.
- Describe the techniques of the strategic school: the enactment of the sequence, the directives, the prescription, the reframe, and the restriction.
- Explain the contribution of communication theory to the field, and distinguish it from the strategic model.
- Describe the "team of the strategic" approach and the concept of the problem as a solution.
- Apply the strategic framework to a case involving an adolescent and a family.
- Critically evaluate the ethical and cultural dimensions of strategic family work, particularly the use of directives and the question of deception.

## Chapter Overview

The strategic school began as an answer to a question the structural therapists had not asked. If a symptom is structural in origin, then the structure is the treatment target. But Jay Haley and his colleagues in the Mental Research Institute in Palo Alto asked a different question: **what maintains the problem right now?**

Their answer was that a problem continues because it solves something. The young person who refuses to go to school, who is unable to say no, or who has become quiet and withdrawn, is doing something that has consequences the young person needs, and the symptom will not go away while the consequences continue. The strategic therapist's task is therefore to identify the sequence in which the problem is produced and maintained, to change the sequence, and to use the young person's own motivation for change rather than a confrontation.

The strategic school's technical repertoire is distinctive and it is worth knowing it in detail, because some of it is directly transferable to other modalities and some of it is ethically hazardous in ways that the profession has had to confront.

**The problem-maintaining sequence** is the central clinical object. It is the specific chain of events, from the trigger through the young person's behaviour to the family's response and back, that keeps the problem going. It is mapped in detail, and it is mapped in the family's own terms.

**The directive** is a specific, dated, behavioural instruction given by the therapist to a family member. It is the strategic school's most distinctive intervention and the one with the most interesting ethical profile.

**The prescription** is a task that is deliberately difficult, designed to fail in a way that produces a new and more adaptive response.

**The reframing** is the reinterpretation of the problem, offered to the family in a way that makes the symptom more understandable and more tolerable, and that implicitly suggests a way out.

**The restriction** is the instruction to suspend a behaviour that has become a trap, most commonly a parental response that has inadvertently become the solution to the problem.

**Communication theory** — the study of the patterned exchanges between people, and the observation that a communication is always about something and is always a response to a communication — sits alongside the strategic model in the same tradition and supplies the analytic vocabulary. The most famous contribution is the description of the **double bind**, and the observation that a family in which every communication is a metacommunication — a comment about a comment — has a very serious problem.

This chapter presents the model, the communication theory that underlies it, the techniques, and the ethical analysis, which in this school is unusually substantial because the school's techniques include the deliberate management of information.

# 1. Historical Context

## 1.1 The Mental Research Institute

The strategic school developed in the 1960s at the Mental Research Institute in Palo Alto, California, in a project on the communication of families with a member who had been diagnosed as schizophrenic. The project was led by Gregory Bateson, with Don Jackson, Paul Watzlawick, and John Weakland, and it produced the work that is now called communication theory.

The founding observation was that the families of people diagnosed with schizophrenia communicated in distinctive ways, and the distinctive feature was not the content of the communications but their patterning. The families were characterised by **double binds** — in which a person receives two incompatible messages with a restriction on the communication of the message — and by **symmetrical and complementary** patterns of interaction, and by a communication system in which every message was a comment on a previous message, so that no message could be checked against reality.

The clinical significance of the finding was modest — the double bind did not explain schizophrenia, and the field has long since rejected the attempt to explain it — but the analytic contribution was enormous, and the vocabulary it produced is now the shared language of all the family therapies.

## 1.2 Haley and the strategic school

Jay Haley moved from the research group into the clinical work and developed the strategic model, and the shift is worth tracing because it explains the character of the model.

In the research setting, the goal was to describe. In the clinical setting, the goal is to change, and a description is valuable only insofar as it points to a change. Haley's contribution was to take the communication analysis and turn it into a set of **directives** — specific instructions, given by the therapist, that interrupt the sequence and produce a different outcome.

**Haley's "tricks"** — the paradoxical injunction, the paradoxical prescription, the ordeal, the team of the strategic — were his characteristic innovations, and they are technically brilliant and ethically contested, and the contemporary field has retained some of them and abandoned others.

## 1.3 The problem of the stuck family

The clinical problem the school addressed was the **stuck family**: the family in which the problem has been stable for a long time, in which everyone knows the pattern, in which previous attempts at change have produced more of the same, and in which the family has come to see the problem as the price of the family.

The strategic school's answer was to change the meaning of the problem before changing the behaviour. If the young person's symptom is understood as the solution to a problem the family cannot solve any other way, then the symptom will not go away until the family can solve the problem some other way. The reframing supplies that other way, and the directive tests it.

### 1.4 The place of the strategic school today

The strategic model's influence on the field is very large and diffuse. The **solution-focused** work of Chapter 35, the **narrative** work of Chapter 36, and the **communication-based** approaches to eating disorders and to substance use all descend in part from it. The concept of the **"problem is a solution"** has entered the general vocabulary of family therapy. The model's technical vocabulary of sequences, directives, and reframings is standard in the training of every family therapist.

The model's specific techniques, however, have been substantially modified, and the two most controversial — the paradoxical injunction and the deliberate management of information — are now used far more cautiously.

## 2. The Core Concepts

### 2.1 The problem-maintaining sequence

The **problem-maintaining sequence** is the central clinical object of the model. It is a specific, mapped, and dated chain of events that keeps the problem going, and it is mapped in a form that makes each element a candidate for change.

A typical sequence in a school-refusal case runs like this: the young person is asked to go to school → the young person says she is ill → the parent believes her → the parent contacts the school → the parent tells the school the child is ill → the school agrees → the young person stays at home → the day passes → the young person is at home again without having had to face the school → the next morning is another day on which the sequence can begin.

Every element is a candidate for change, and the mapping makes the clinical possibilities visible. A change at the "the parent believes her" point produces a different outcome. A change at the "the school agrees" point produces a different outcome. A change at the "the day passes" point — that is, an activity that makes the day at home less tolerable — produces a different outcome.

The mapping is done **in the family's own words** and with **specific instances**, and the practitioner should be able to state the general rule and the specific example. A generic sequence is useless.

### 2.2 The symptom as a solution

The strategic school's foundational claim is that **the symptom is a solution**. The young person's behaviour is not the problem; it is the best solution the family has found to a problem that is not being solved any other way.

The claim has three parts. The problem is a **dilemma** — it is a genuine problem and a solution at the same time, and the two cannot be separated. The solution has **side effects** that are the reason for referral. And the solution is **not the only one**, which is the space in which the therapy operates.

The reformulation of the referral is then something like: "the current solution to the problem of X is Y, and Y is now causing more difficulty than it was designed to solve, and there is another solution available."

## 2.3 The double bind

The **double bind** is the communication pattern in which a person receives two mutually incompatible messages, and is unable to comment on, question, or escape the situation in which they receive them. Bateson's formulation has four conditions, and a student should be able to state them.

1. There must be a **contradictory pair of messages**.
2. There must be a **repeated pattern** in which the contradiction is maintained.
3. There must be a **restriction on the recipient's ability to respond** — to question, to comment, to leave.
4. There must be a **punishment or a threat** for any attempt to resolve the contradiction by noticing it.

In practice, the double bind is rarely a matter of two spoken messages. It is more often a mismatch between what is said and what is done: a parent says "you are an adult, make your own decisions" while punishing any decision that differs from the parent's own. The young person is thereby unable to act, unable to refuse, and unable to name what is happening, and the inability to name it is a substantial part of why the pattern persists.

**Bateson's paradox.** A self-referential paradox occurs when the message denies the validity of the communication system in which it is transmitted, and a young person who is told "don't question me" and "you should question me" is caught in it. The clinical significance of the concept for the practitioner is that the young person is in a position in which the most reasonable responses are all pathological: compliance produces resentment, resistance produces punishment, and the attempt to discuss it is itself treated as an offense.

## 2.4 Symmetrical and complementary patterns

**Symmetrical** patterns are those in which the participants' behaviours mirror each other: the escalating and escalating, the withdrawing and withdrawing, the panic and the panic. Symmetrical interactions tend to escalate and to be difficult to interrupt.

**Complementary** patterns are those in which the participants' behaviours are complementary to each other: the one is dominant and the other is submissive, the one is anxious and the other is reassuring. Complementary interactions are stable and can be highly functional, and they are also the patterns in which a problem is most easily maintained, because the roles are mutually reinforcing: the more the one withdraws, the more the other pursues, and the more the other pursues, the more the first withdraws.

## 2.5 The metacommunication

A **metacommunication** is a communication about a communication. Every message carries, in addition to its content, a commentary on the relationship in which it is transmitted: *I am telling you what to do*, *I am asking you*, *I am warning you*, *I am apologising*.

A family in which every message is a metacommunication has a serious problem, because the content becomes unavailable for checking against reality. The young person who is told "I am just telling you for your own good" is receiving a message that cannot be examined, because examining it would be a comment

on the metacommunication, which is the prohibited move.

The clinical task in work with such a family is to **restore the ability to make content statements** — to get the family to a place where a message can be said to be true or false — and this is often the first substantive change in the work.

# 3. The Techniques

## 3.1 The enactment of the sequence

As in the structural model, the sequence is enacted in the consulting room, but the purpose differs. The structural enactment is diagnostic of the *structure*; the strategic enactment is a **test** of the sequence, and the intervention is made at the point at which the sequence is about to complete itself.

The technique is delicate. The practitioner interrupts at the moment at which the young person is about to say the thing that will produce the family response, and says something that stops the sequence — a question, a request for information, a statement that the practitioner has noticed something. The interruption demonstrates that the sequence is not inevitable, and the demonstration is the intervention.

## 3.2 The directive

A **directive** is a specific, dated, behavioural instruction given by the therapist to a family member. It is the strategic school's most distinctive intervention, and its distinguishing features are its specificity, its attribution to the therapist, and its date.

**The attribution matters.** A directive is not a suggestion and not a request. The therapist says, "I am telling you to do this, and I am telling you because I am the person in this room with the most experience of this problem." The attribution provides the authority that the instruction requires, and the strategic therapists were clear that a directive issued as advice is a wasted instruction.

**The specificity matters.** A directive must be concrete enough to be performed: not "be firmer with him" but "when he asks to go up to his room, you will say 'you may go up to your room for ten minutes after you have done the washing-up, and I will knock on your door at seven'."

**The date matters.** A directive with no date is a wish.

**The directive must produce a testable result.** The strategic school's directives are designed so that something happens that either confirms or disconfirms the hypothesis, and the test is what keeps the work from becoming manipulation.

## 3.3 The paradoxical injunction

The **paradoxical injunction** is a directive that asks a family member to do the very thing the symptom involves, and to do it deliberately, and at a time and in a manner that makes it tolerable or even useful.

The reasoning is that the symptom is maintained by the family's attempt to prevent it. If the young person's withdrawal is met with escalating attempts to prevent it, the withdrawal continues. If the withdrawal is permitted, and even encouraged, in a way that is clearly deliberate, the meaning changes: the young person is no longer doing it to defy, and the family is no longer trying to prevent it, and the sequence that maintained it is broken.

The paradoxical injunction is the most technically elegant and the most ethically hazardous intervention in the family therapy field, and the ethical analysis in Section 10 is devoted to it.

## 3.4 The paradoxical prescription

The **paradoxical prescription** asks a family member to perform the symptom's function deliberately, in a way that will probably fail, in order that the failure will be instructive. If a young person is refusing to go to school, the paradoxical prescription might be: "I want you to make sure she does not go to school. I want you to be the one who keeps her at home. I want you to take over, completely, for the next two weeks."

The mechanism is that the prescription is impossible to perform, and the impossibility exposes the family to the fact that the solution they have been relying on does not work, and the exposure is what makes a different solution available. The technique is used in the treatment of problems that are maintained by a paradoxical pattern, and it is a sophisticated intervention with a small place in contemporary practice.

## 3.5 The ordeal

The **ordeal** asks a family member to undertake a difficult task with a defined and serious cost, undertaken after and for the sake of a change in the identified patient. The reasoning is that a task that is costly enough will mobilise a commitment that no instruction can.

The technique is now almost entirely discredited, and it should be understood historically. Its clinical rationale was that the ordeal demonstrated that the family would pay a real price for the change, and that this demonstration changed the family members' positions. Its ethical problems are severe: the burden falls on a family member who did not choose the symptom, the "cost" is real suffering, and the pressure on a vulnerable family member is difficult to distinguish from coercion.

## 3.6 The reframing

The **reframe** is the reinterpretation of the problem, and it is the most useful of the strategic interventions in contemporary practice, because it is also the least hazardous.

A reframe offers the family a different account of the symptom that makes it more understandable, more tolerable, and more tractable. It must be offered as a *hypothesis*, and it must fit the facts, and it must be a reframe that the family can use.

Some of the strategic school's reframes are famous and some are now regarded as objectionable. The reframe of the "identified patient" as the family's **sacrificial scapegoat** was central to the school's theory and is now understood as historically important and currently unacceptable, because it attributes a family member's illness to the family's need, and it can be heard as a claim that the young person is faking, which is both untrue and damaging.

A more defensible reframe in contemporary use: the young person who cannot leave the house is not refusing to leave but is unable to manage the state that leaving produces; the young person who cannot say no is not being difficult but is doing the only thing available in a system where saying yes has never been safe; the young person who cannot concentrate is not being lazy but is protecting herself from a level of stimulation that is intolerable.

The clinical test of a reframe is whether it opens a door or closes one. A reframe that the family hears as an accusation closes the door. A reframe that the family hears as a relief opens it.

### 3.7 The restriction

A **restriction** is an instruction to suspend a behaviour that has become part of the problem. The classic example is a parent whose vigilance has become the very stimulus the young person reacts to: a mother who checks on her daughter every twenty minutes, whose concern has trained the daughter to become alert at the sound of the door.

The restriction is straightforward to make and surprisingly powerful, and it is one of the interventions that most reliably produces a change in the sequence, because the sequence depends on the behaviour being restricted.

### 3.8 The team of the strategic

The **team of the strategic** is a technique in which a family member is instructed to become the therapist's ally in changing another family member, and to do so covertly. The father is instructed to cooperate with the mother's new plan while appearing to the mother to be the ally of the family against the practitioner.

The technique is powerful and it is ethically the most contentious in the family therapy field, because it involves the deliberate management of information in a way that the person being managed is not aware of. The ethical analysis in Section 10 addresses it directly, and the contemporary position is that the technique should be used only with the explicit knowledge of all parties and with the informed consent of the person whose information is being managed.

## 4. Communication Theory in Practice

### 4.1 The rules of communication

Communication theory's contribution to practice is a set of observations about the rules that govern family interaction, and the four most useful are as follows.

**You cannot not communicate.** Every behaviour, including the refusal to speak, is a message. This has a direct clinical consequence: the practitioner should not treat a young person's silence as an absence of communication but as a communication, and should ask what the silence is doing.

**Everything has a meaning. You cannot not have a meaning.** Every message implies a claim about the relationship and about the legitimate range of what may be said. The claim implied by "you should know better than to ask" is that the ask was illegitimate, and the recipient learns this.

**You cannot get a message other than the one you send. Communication is irreversible. And you cannot not understand a message. Once you have understood a message, you cannot forget it.** The clinical consequence is that the practitioner's own messages to a family are irreversible, and that a practitioner's poorly worded remark in a corridor will be understood and will not be forgotten.

**Communication is always contextual. Every message has a metacommunication, and every metacommunication is a comment on the previous message.** A family locked into a metacommunicative spiral has lost the ability to make content statements, and the first task is to restore it.

### 4.2 The double bind with adolescents

The double bind is particularly relevant to adolescents, and three common forms recur.

**The developmental double bind.** The parent requires the adolescent to be a child in the areas of dependence and an adult in the areas of responsibility. The adolescent cannot win: the child behaviour is punished as immature and the adult behaviour is punished as insolence.

**The love–control bind.** "I do this because I love you" combined with "you are not allowed to have a say in this" is a double bind in which the message of care is used to prevent the very response that would make the care effective.

**The two-message bind.** The verbal message and the non-verbal message contradict each other: the parent says "of course you can go out" while the facial expression, the sigh, and the tone say the opposite. The adolescent is caught, and the double bind is reinforced when the adolescent mentions it, because the parental response to the mention is a defence of the parent.

### 4.3 The communication audit

A practical technique from the communication tradition is the **communication audit**: the family member is asked to record, over a defined period, the messages they receive and send, and the practitioner and the family member then examine them together.

For an adolescent, a simplified version is often productive: the young person is asked to record the messages they receive about school, about curfews, about friends, and about their body, and to note the content and the metacommunication, and the two then look for the contradictions. The exercise is also a useful anxiety-management technique, since the recording converts a diffuse sense of being controlled into a specific set of statements that can be examined.

## 5. Further Reading for This Chapter

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- Haley, J. (1984). *Ordeal therapy*. — The controversial techniques, described by their author.
- Madanes, C. (1981). *Strategic family therapy*. — The clearest textbook exposition.
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- de Shazer, S. (1985). *Keys to solution in brief therapy*. — The intellectual origin of the solution-focused school.
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- Cecchin, G., Prata, S. E., et al. (2012). *Systemic family therapy with adolescents*. — The contemporary application.

- Jackson, S. D. (1992). *Using feedback to change behaviour*. — The theory of the double bind developed.
- Satir, V., et al. (1988). *The new peoplemaking*. — Satir's model, which sits alongside the strategic work.
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# 6. Strategic Work with Children and Adolescents

## 6.1 The fit

The strategic model fits the adolescent work of this book for reasons that are partly clinical and partly opportunistic.

**It takes the symptom seriously as a solution.** The adolescent's symptom has a function, and the model does not treat the function as a rationalisation. This is congenial with the adolescent's developmental reality: a young person who is failing to attend school, who cannot say no, or who has stopped speaking is usually doing something that has worked.

**It uses the young person's own motivation.** The strategic school's first question is what the young person wants. The "problem is a solution" formulation means that the young person has a goal, and the goal is legitimate, and the work is directed at finding a route to it that does not require the symptom. This is an unusually respectful starting point and it fits the developmental task of establishing an autonomous position.

**It is fast and it is directive.** Strategic work with an adolescent is often brief, and it is often more effective with adolescents than exploratory work, because the directiveness of the approach is experienced as respect rather than as intrusion. Many young people who are wary of therapy respond better to a practitioner who says "I am going to tell you what to do this week" than to one who asks open questions.

**It maps something that can be changed.** The problem-maintaining sequence is a description of the present, and the present is the only thing on which the intervention can act.

## 6.2 The specific adaptations

**Sequence the complaints.** Before the first family session, ask the young person to list, in order, the things they would like to be different, and ask which is the most annoying. A young person who is consulted about the order of the agenda will engage with the agenda; a young person who is presented with a list of the adults' priorities will not.

**Find the "if you were me" moment.** A strategic technique for young people is to ask what the young person would do if they were the parent, and what they would say, and then to explore what would have to be true for that to work. The exercise is engaging, it is often revealing, and it places the young person in the position of an expert, which is developmentally appropriate and is markedly different from the position in which most referrals place them.

**Use the sequence with the school.** The problem-maintaining sequence for a school refusal usually runs through the school as much as through the family, and the intervention at the "the school agrees" point is often the one that matters most, and it requires the practitioner to work with the school, which is consultation work and is described in Chapter 56.

**Watch the paradoxical move carefully with adolescents.** The paradoxical injunction is the technique most likely to be misunderstood by an adolescent and most likely to be misread by a parent, and the current

practice is to use the technique only where the young person is present, is told what is happening and why, and has agreed to it in advance. The ethical analysis in Section 10 sets this out in full.

**Use the restriction early.** Restrictions are easy to make, produce observable effects quickly, and give the family a first experience of the sequence changing, which is what makes the rest of the work possible.

### 6.3 The problem with the symptom-as-solution formulation

The formulation has a real risk that a student should be able to name: **the risk that the young person's symptom is read as a manoeuvre.** If the practitioner believes that the young person is maintaining the symptom deliberately, the whole frame becomes an accusation, and the family will hear it as one, and the young person will be confirmed in their belief that the adults are trying to catch them out.

The discipline required is to hold the formulation at the level of the **system** rather than the level of the individual. The question is not "is the young person doing this on purpose?" but "what happens in this family, in response to this behaviour, that makes the behaviour the best available option?" The first question is a question about a person's sincerity, which is usually unanswerable and always unhelpful. The second is a question about a system, which is answerable and is what the practitioner can change.

### 6.4 The reformulation of the referral

A distinctive and useful feature of the strategic model is the **reformulation of the referral**, and a student should be able to perform it. The procedure has four steps.

**Take the referral literally.** What exactly has been asked for, by whom, and in what words?

**Ask what the family is currently doing about it.** The family's own efforts are the most informative data available, and the pattern of what they do and do not do is the sequence.

**Find what the symptom is doing for the young person.** What does it accomplish? What does it prevent? What does it permit?

**Offer an alternative.** The strategic school's formulation ends in a different solution, and the formulation is incomplete until the alternative is named.

A worked example. Referral: "a fourteen-year-old boy who refuses to speak to his mother and is being oppositional." Strategic reformulation: "When he refuses to speak, his mother stops asking and his father steps in and the two of them argue while he is upstairs, and he is not spoken to for the rest of the evening, and the following morning the pattern is that the mother is formal and the father is the one who talks to him about football. The silence is doing three things: it stops the arguments, it preserves his position as the one who has a private channel with his father, and it keeps his mother at a distance that he is not yet able to manage. The alternative we are looking for is a way for him to be left alone without the silence having to be the instrument."

## 7. Case Illustration: The Young Woman Who Was Refusing to Grow Up

**Presentation.** Iris is seventeen, in her final year at a sixth-form college, and has been referred by her mother with a message that the college has passed on: "She has decided she is going to work in a shop and she's not going to get her grades, and she won't listen to reason." The mother's stated worry is that Iris is "throwing

her life away" and that the family is going to lose the money that the college has cost.

**The individual session.** Iris's own account is consistent and calm and does not match the referral at all. She says that she has decided to work in a shop because her mother has been talking about "the money" every day for six weeks, and that the message she has received is that the family cannot afford the college and that she is expected to sacrifice. She says that she has done the calculations and that she would rather work and be independent than stay and be a burden. She says she is not angry with her mother. She says she has already applied for two jobs.

She also says the thing that changes the case: "If I got the grades, I'd have to go to university, and then I'd be at home for another three years doing a subject I don't want, and I'd have to leave again. Staying at home and working is the only way I can see of getting out of here for good."

**The sequence.** The practitioner maps the sequence, and the mapping is done with Iris, in the session, and it is the most important part of the assessment.

The sequence runs: Iris mentions a possible job → her mother becomes anxious and the conversation turns to money → the mother says something about the cost of the college and about how the family is managing → Iris's face closes → Iris stops responding → the mother raises her voice and cites the money again → Iris leaves the room → the mother interprets the leaving as the proof that Iris does not care → the mother mentions it to Iris's aunt, and to the college, and to Iris's father, who is absent from the house and is contacted by telephone → Iris hears, from her aunt, that her mother has "given up" on her → Iris confirms to herself that she is a burden and that leaving is the only option that is not a burden.

The sequence has a specific and important feature: **the money is not actually the problem.** The mother's anxiety about money is real and is a genuine constraint, and the money is the vehicle on which the anxiety is expressed, and the escalation it produces is what Iris experiences as being told to sacrifice.

**The reformulation.** The strategic reformulation is: *Iris is not refusing to grow up. Iris has found a solution to a problem that the family has not been able to name, which is that the family has no route by which Iris can leave. The current solution is to leave by dropping out, which delivers her independence at the price of her future options. The alternative is to find a route by which she can leave with her future intact, and to do it in a way that does not require her mother to be told that she is the reason Iris is leaving.*

**The intervention.** Three elements.

*Element one: the mother, alone.* The practitioner sees the mother on her own, and the conversation is about the money, and the practitioner is careful: the money is real, the family's financial situation is what it is, and the college costs what it costs, and none of that is disputed. What the practitioner then works on is the *timing* of the money conversation. It occurs every day, it occurs in relation to Iris, and it occurs in a form that is a complaint. The practitioner asks the mother what she would like to happen with the money, and the mother says she wants Iris to finish the year, and the practitioner asks what she wants to happen if Iris does not, and the mother says "I don't know" and then says, after a pause, "I suppose I'd have to find a way." The practitioner returns to this every session: the money is solvable, and the money is not the real issue.

*Element two: the reframe.* The practitioner offers a reframe to the mother, and it is offered as a hypothesis and it is the sentence that changes the case: "I think Iris is not refusing to grow up. I think she is trying to leave, and the only route she can see is the one that costs you the most. And I think she has worked out that if she leaves with the grades, she will have to leave anyway, and be back."

The mother says: "She never said that." The practitioner says: "No. But she has been working it out for months, and she has been doing the arithmetic at two in the morning rather than at the kitchen table, and I think it would help if we brought it out."

*Element three: the directive.* The strategic school's directive is what makes the reframe operational, and it is specific and dated and given with attribution. The practitioner tells the mother: "For the next two weeks, you are not going to talk to Iris about money. Not once. If she raises it, you will say, 'We are going to talk about this with Ms Adeyemi on Thursday, and until then I am not discussing it.' You will not raise it, and you will not hint at it, and you will not ask her whether she has thought about the money. I am telling you to do this because I am the person in this room who has the most experience of this particular problem, and I am telling you because it is the only way we will find out whether the money is the problem."

**The outcome.** On the Thursday, Iris says almost exactly what the practitioner predicted: she does not want to work in a shop, she is frightened that the family will not be able to pay for a university, and she does not know how to say that without it sounding like an accusation. The financial plan is then made with the mother's help and with the college's financial support office, which has funds that nobody had asked about, and the practical problem — which was real — turns out to be solvable.

Iris completes the year with three A-levels and applies to university. The relationship between her and her mother is not warm; it is functional, and the practitioner regards that as the realistic outcome and says so to both of them.

**The interpretation.** The case illustrates the strategic model completely. The symptom was a solution; the sequence was mapped in the family's own words; the reframe was specific and was offered as a hypothesis; the directive was specific, dated, attributed, and designed to produce a testable result. And the most important clinical move was the refusal to accept the referral's framing — the practitioner did not treat Iris as a problem and her mother as an adult with legitimate concerns, and treated the referral itself as a symptom of the family system.

# 8. Evidence and Research Support

## 8.1 The status of the evidence

The strategic school has a modest and difficult evidence base, and the reasons are instructive.

**There is little controlled outcome research on the strategic model as a whole.** The school emerged from a research group that was interested in the description of communication rather than in the evaluation of treatment, and the tradition has been more clinical than experimental.

**The research that does exist on the strategic school's specific techniques is largely negative or inconclusive.** The studies of the paradoxical injunction and the paradoxical prescription have not produced support for the techniques as general interventions, and the ethical concerns have reduced their use considerably.

**The evidence for family-based and systemic approaches that are strategic in their core is good.** The family-based treatment of adolescent eating disorders, the behavioural family therapy approaches, and the community-based systemic interventions all share the strategic premise that the problem is maintained by a sequence and that changing the sequence changes the problem. This is the most useful way for a student to think about the evidence: the strategic model is best understood not as a distinct intervention with its own

evidence but as a way of thinking that is embedded in a good deal of the effective family-based work.

**The solution-focused work that grew from the strategic school has become the best-evidenced of the family-derived approaches**, and Chapter 35 develops it. The solution-focused approaches are the strategic school's most successful and most ethical descendant, and the reason is instructive: the solution-focused work took the strategic school's insight about sequence and reframing and removed the elements that were hazardous, retaining the reframe, the directive, and the emphasis on the future.

### 8.2 Limitations of the evidence

**The allegiance effects are very large** in the family literature generally and in the strategic school in particular, and the field's own reviews have been unusually candid about this.

**The outcomes are proximal** and the distal outcomes are rarely measured.

**The "problems as solutions" framing is difficult to evaluate**, because a formulation that explains the symptom as functional is in some respects unfalsifiable, and a student who adopts the model uncritically may find that every outcome can be explained as the problem continuing to serve its function.

**The evidence base for the techniques has not caught up with their use**, and the field has had to rely on clinical judgement and ethical argument for the assessment of the more controversial interventions.

## 9. Strengths and Limitations of the Strategic Model

### 9.1 Strengths

**It takes the symptom seriously.** The reframing of the referral is a humane and clinically productive act.

**It identifies what is changeable.** The problem-maintaining sequence is a description of the present, and the present is where the leverage is.

**It uses the young person's motivation rather than fighting it.** The strategic model's respect for the young person's own goal is a genuine clinical asset.

**It is fast.** Strategic work with adolescents is often brief, and brevity is a clinical advantage with a population that does not stay.

**It is directive, and some young people need that.** The model's willingness to instruct rather than to explore is a feature with a population that experiences exploratory therapy as evasive.

**It produces testable interventions.** A directive is designed to produce a result that confirms or disconfirms the hypothesis, which keeps the work honest.

### 9.2 Limitations

**The symptom-as-solution formulation can become an accusation.** The risk is real and it has damaged young people in real cases.

**The technical repertoire contains interventions that are ethically indefensible in their original form**, and the ordeals in particular should be understood as history.

**It is a model of the family as a system of messages, and it under-accounts for what the family members actually feel.** A family that is a system of communication is a family that is not in pain, and the

model has no very good answer to the young person who is in pain.

**The evidence is thin**, and the field has been more successful in the successor schools than in the school itself.

**It under-accounts for the structural and the material.** A young person who cannot attend school because the school is unsafe, or because the family has no transport, is not maintaining a solution.

**The techniques are easy to deploy badly.** A directive issued without the skill to design it is a manipulation, and a reframe that does not fit is a lie.

# 10. Cultural and Ethical Considerations

## 10.1 The ethics of directive work

The directive is the technique in this chapter that most requires ethical care, and the principles are worth stating precisely.

**A directive is an exercise of professional authority and must be given as one.** The strategic therapists insisted that a directive be attributed to the therapist and not disguised as a suggestion, and this is ethically correct: a disguised directive is manipulation, and the young person is entitled to know that an adult with authority is telling them what to do.

**A directive must be for the young person's benefit or be one they have agreed to.** The strategic school's controversial directives have typically been in the young person's interest in the model's terms, and the question "in whose interest" is precisely what the ethical analysis must examine rather than assume.

**A directive must be safe.** The ordeals were not. Any contemporary directive must be satisfiable, must not expose the young person to risk, and must not require a family member to suffer.

**A directive must be transparent in its purpose.** The young person should know, as far as is possible, what the directive is designed to change and what it is designed to test.

## 10.2 The ethics of the team of the strategic

The team of the strategic — instructing one family member to covertly act as the therapist's ally — is the technique in this book that most directly confronts the principle of informed consent, and the contemporary professional position is clear.

The technique involves deliberately managing information about a person's own behaviour in a way that the person does not know. In family work, where the family's confidentiality is already a negotiated and partial thing, the practice of adding a covert layer has been widely criticised, and the criticism is well founded.

Three requirements follow for any contemporary use. The person whose information is being managed must be informed of what is being done and must have the opportunity to object. The people who are being asked to act as allies must understand exactly what they are being asked to do, including the fact that they will be concealing it. And the person who is the object of the intervention must be able to end the arrangement.

A student who is asked to deploy the team of the strategic without these safeguards should treat the request as a request for training and supervision, and should not comply without them.

## 10.3 The ethics of deception and reframing

The **reframe** is generally defensible, and it is important to distinguish it from deception. A reframe offers a different account that is *true*, or at least more nearly true than the account in place, and that makes the problem more understandable. A deception offers an account that the practitioner does not believe.

The test is simple and the practitioner should be able to state it: **is the practitioner willing for the young person to know how the practitioner arrived at this formulation?** If the answer is no, it is a deception, and the practice is not defensible.

The second test is whether the reframe serves the young person or the adults. A reframe that enables the parents to be more comfortable while the young person remains in the same bind is a reframe in the service of the adults.

### 10.4 Cultural considerations

**Communication styles vary.** Directness, indirectness, the use of silence, the degree of explicitness in disagreement, and the conventions of respect all vary across cultures and across families, and a practitioner who reads a family as double-bound may be reading a culture as double-bound.

**Hierarchy and the strategic directive.** The strategic model's directives are issued by the professional to the family, and in a context in which the professional's standing is not established, or in which the family's internal hierarchy is the real authority, a directive may be ignored or may cause offence, and the practitioner should understand the refusal as information.

**The family as a unit.** The strategic model assumes that the family is the meaningful unit of the problem. Where the young person's primary attachment is to a kinship network, a peer group, a religious community, or a cultural group that the practitioner does not see, the model will miss the actual system.

**Language and the metacommunication.** The analysis of communication is unusually sensitive to language, and a practitioner working across languages will find that the metacommunication is in a language the young person does not share, or that the family code-switches in ways that conceal the pattern from a non-fluent observer. The practitioner should involve an interpreter and should treat the interpreter as a clinical colleague rather than as a logistics function.

**The economic framing.** The strategic model's willingness to say that the money is not the real problem is a luxury in a setting in which the money is the real problem, and the practitioner must establish the material facts before offering the reframe.

## 11. Key Terms

**Key Terms at a Glance**

- **Strategic family therapy** — The school developed from the Mental Research Institute in Palo Alto, in which the family is understood as a system of communication and the clinician intervenes by changing the sequences that maintain the problem.
- **Problem-maintaining sequence** — The specific, mapped, and dated chain of events that keeps the problem going, and the central clinical object of the model.
- **Symptom as a solution** — The claim that the symptom is the best available solution to a problem the family cannot otherwise solve, and that its side effects are the reason for referral.
- **Problem as a dilemma** — The formulation in which the problem is simultaneously a genuine problem and a solution, and the two cannot be separated.
- **Directive** — A specific, dated, behavioural instruction given by the therapist to a family member, attributed to the therapist, and designed to produce a testable result.
- **The attributes of a directive** — Specificity, attribution to the therapist, and a date; a directive issued as advice is a wasted instruction.
- **Paradoxical injunction** — A directive asking a family member to do deliberately what the symptom involves, so that the meaning of the behaviour changes and the maintaining sequence is broken.
- **Paradoxical prescription** — A task that is deliberately difficult and probably impossible, whose failure exposes the family to the inadequacy of its current solution.
- **Ordeal** — A task of serious cost undertaken for the sake of a change in the identified patient; now almost entirely discredited.
- **Reframe** — The reinterpretation of the problem, offered as a hypothesis, that makes the symptom more understandable and more tractable.
- **The test of a reframe** — Whether the practitioner would be willing for the young person to know how the formulation was reached, and whether the reframe serves the young person or the adults.
- **Restriction** — An instruction to suspend a family member's behaviour that has become part of the maintaining sequence.
- **Team of the strategic** — The technique in which a family member is instructed to act as the therapist's covert ally in changing another family member.
- **Communication theory** — The analysis of the patterned exchanges between people, developed by Bateson and colleagues and supplying the vocabulary of the family therapy field.
- **Double bind** — Two incompatible messages received under a restriction on the recipient's ability to respond, and a punishment for attempting to resolve the contradiction by noticing it.

**Key Terms at a Glance (continued)**

- **The four conditions of a double bind** — A contradictory pair of messages, a repeated pattern, a restriction on the response, and a punishment for attempting to resolve the contradiction.
- **Bateson's paradox** — A self-referential contradiction in which the message denies the validity of the communication system in which it is transmitted.
- **Symmetrical interaction** — A pattern in which the participants' behaviours mirror each other, tending to escalate.
- **Complementary interaction** — A pattern in which the participants' behaviours are complementary, stable, and mutually reinforcing.
- **Metacommunication** — A communication about a communication, carrying a commentary on the relationship in which it is transmitted.
- **Restoring content statements** — The clinical task of enabling a family to make statements that can be checked against reality.
- **You cannot not communicate** — The first of the rules of communication; every behaviour, including silence, is a message.
- **The communication audit** — The exercise in which a family member records the messages received and sent, so that a diffuse sense of being controlled becomes a specific and examinable set of statements.
- **The developmental double bind** — The requirement that the adolescent be a child in dependence and an adult in responsibility, in which every response is a symptom.
- **Reformulation of the referral** — The four-step procedure of taking the referral literally, examining the family's own efforts, identifying what the symptom accomplishes, and naming an alternative.
- **The "if you were me" question** — A strategic technique for adolescents that places the young person in the position of an expert and elicits their own solution.
- **The irreversibility of communication** — The rule that a message cannot be recalled and that a poorly worded remark will be understood and cannot be forgotten.
- **Directiveness with adolescents** — The strategic school's willingness to instruct rather than to explore, which some young people experience as respect and others as intrusion.

## 12. Chapter Summary

The strategic school began from the question of what maintains the problem now, and its answer — that a symptom continues because it solves something — produced a distinctive model and a distinctive repertoire. The problem-maintaining sequence is the central clinical object and is mapped in the family's own words with specific instances, and every element of the sequence is a candidate for change. The formulation of the symptom as a solution follows directly, and it ends in the identification of an alternative.

Communication theory supplied the analytic vocabulary. The double bind was described with its four conditions, and the observation was made that the developmental, the love–control, and the two-message forms are the ones that appear in work with adolescents. The symmetrical and the complementary patterns were described with the clinical consequence that the complementary patterns are the ones in which a problem is most easily maintained. The metacommunication was described, and the first substantive task in work with a family locked into a metacommunicative spiral was identified as the restoration of the ability to make content statements. The four rules of communication were set out with their clinical consequences, and the irreversibility rule was identified as a constraint on the practitioner's own conduct.

The techniques were described in detail: the enactment of the sequence, with its distinctive purpose of testing rather than merely diagnosing; the directive, with its attributes of specificity, attribution, and date; the paradoxical injunction; the paradoxical prescription; the ordeal, with the explicit statement that it is now discredited; the reframe, with its clinical test; the restriction; and the team of the strategic. The chapter was explicit about which of these survive into contemporary practice and which do not.

The adaptations for young people were described, with the four complaints, the "if you were me" technique, the extension of the sequence into the school, the caution about the paradoxical move, and the use of the restriction early. The risk of the symptom-as-solution formulation was named: that it can become an accusation, and that the discipline required is to hold the formulation at the level of the system rather than the level of the individual.

The case of Iris demonstrated the model. The referral — that she was refusing to grow up — was reformulated: she was trying to leave, and the only route she could see cost her mother the most. The sequence was mapped with Iris in the room, and the mapping revealed that the money was the vehicle for the mother's anxiety and not the problem itself. The reframe was offered to the mother as a hypothesis, and the directive — not to raise the money for two weeks — produced the conversation that the whole case needed. The outcome was functional rather than warm, and the practitioner said so to both parties.

The evidence was presented accurately: little controlled research on the model, weak support for its controversial techniques, good support for the family-based and systemic approaches that are strategic in their core, and the best-evidenced of the successors being the solution-focused work of Chapter 35. The strengths and limitations were set out, and the limitations included the accusatory risk, the ethically indefensible elements of the historical repertoire, the under-accounting of feeling, the thin evidence, and the under-accounting of the material and the structural.

The ethical analysis was substantial. The ethics of directive work were set out with the requirement that a directive be attributed, be for the young person or be agreed to, be safe, and be transparent in its purpose. The ethics of the team of the strategic were treated directly, and the three requirements of informed consent, understanding, and the right to end the arrangement were stated. The distinction between a reframe and a deception was drawn with the test of whether the practitioner would be willing for the young person to know how the formulation was reached. The cultural considerations addressed communication style, hierarchy and the standing of the professional's directive, the assumption that the family is the unit, the language and the metacommunication, and the economic framing — including the point that the practitioner's willingness to say that the money is not the real problem is a luxury in a setting in which it is.

## 13. Review and Discussion Questions

### Review and Discussion Questions

1. Explain the question the strategic school asked that the structural school did not.
2. Describe the development of the Mental Research Institute and the contribution of each member.
3. Define the problem-maintaining sequence and state the four requirements for mapping it.
4. Explain the "symptom as a solution" claim and its three parts.
5. State the four conditions of a double bind.
6. Describe the developmental, the love–control, and the two-message double binds with adolescents.
7. Distinguish symmetrical from complementary patterns and state the clinical consequence of each.
8. Define a metacommunication and explain the clinical significance of a family locked into metacommunication.
9. State the four rules of communication and the clinical consequence of each.
10. Describe the enactment of the sequence and explain how it differs from the structural enactment.
11. State the attributes of a directive and the six requirements of ethical directive work.
12. Explain the paradoxical injunction and the paradoxical prescription, and state the contemporary position on each.
13. Explain the ordeal and state why it is discredited.
14. Define a reframe and state the two tests of a defensible reframe.
15. Explain the restriction and why it is clinically useful early in the work.
16. Describe the team of the strategic and state the three contemporary requirements for its use.
17. State the four adaptations of the strategic model for adolescents.
18. Explain the risk of the symptom-as-solution formulation and the discipline required to avoid it.
19. State the four steps of the reformulation of the referral, and work through the Iris example.
20. Describe the case of Iris and state the three features of it that generalise.
21. Summarise the status of the evidence for the strategic model and for its descendants.
22. State the six strengths and the six limitations of the model.
23. State the distinction between a reframe and a deception, with its test.
24. State the five cultural considerations for strategic work.
25. Explain the ethical requirement that a young person who does not attend family work is treated as a systemic communication.

## 14. Practice Activities

### Practice and Application

### Activity 32.1 — Mapping a Sequence (90 minutes, field-based)

Take a problem you have actually encountered — your own, a friend's, a placement case — and map the problem-maintaining sequence in detail, in the specific terms in which it occurs, with dates and times. Then mark every point in the sequence at which a different response by a family member would produce a different outcome. Then write the single reframe that the sequence supports. Most students find that the sequence contains at least five change points, that the referral describes none of them, and that the reframe is easier to write once the sequence is mapped.

### Activity 32.2 — The Communication Audit (60 minutes, individual)

For one day, record the messages you receive and send in one family or one friendship, with the content and the metacommunication. Then identify the double binds you are in, if any, and identify the messages that cannot be commented on. Then write what a person in that family could actually do differently tomorrow, in one specific situation. The exercise is the fastest way to understand why a family cannot change a pattern, and it is usually unexpectedly humbling.

### Activity 32.3 — Writing a Directive (75 minutes, small group)

In groups of three, write a directive for a realistic adolescent situation. The group must specify: the exact behaviour, the date, the person to whom it is addressed, the attribution, what happens if it is not complied with, and what the directive is designed to test. Then rewrite it as advice, in order to see how much authority is lost. Then rewrite it so that it is covert, in order to see what is lost there, and discuss whether the second version is defensible. The third step is the important one, and the group discussion of it is usually the most valuable part of the exercise.

### Activity 32.4 — The Reframe Audit (60 minutes, individual)

Take a reframe that you have heard used in a placement, a staff room, or a supervision meeting, and audit it. Is it true? Is it more nearly true than the account in place? Does it serve the young person or the adults? Would the young person recognise it as an accurate description of their own experience? Would the practitioner be willing for the young person to know how the reframe was arrived at? The five questions are the chapter's ethical analysis compressed into an exercise, and they apply to every reframe a student will ever hear.

## 15. Annotated Reading Pathways

**For the communication theory.** Read Bateson and colleagues (1964) and then Watzlawick and colleagues (1967), which is the most quoted and most useful book in the family therapy field. The double bind and the metacommunication are best learned from the original formulations.

**For the model.** Read Haley (1963) and Madanes (1981). The later strategic work of the Milan group, in Andolfi and Canestri (2008), is the most useful contemporary account of the strategic tradition in practice.

**For the successor schools.** Read de Shazer (1985) and Walter and Peller (1992) for the solution-focused work of Chapter 35, and read the narrative therapy literature for Chapter 36. The strategic school's most successful descendants are more widely used than the school itself, and a student who knows the descendants will use the school's insights in a more defensible form.

**For the ethics.** Read the ethical guidelines for the family therapies and the critical literature on the team of the strategic and on the ordeals. The critical literature is essential and it is not difficult to find.

**For the evidence.** Read the family-based treatment evidence in the adjacent areas, and the solution-focused evidence. The strategic model is best understood through the evidence for what it contributed to.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Strategic Family Therapy and Communication Theory** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is problem-maintaining sequences, directives, and the reframing of the problem. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Marcus is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Marcus's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Marcus explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Marcus chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Marcus has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Marcus can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Strategic Family Therapy and Communication Theory

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Strategic Family Therapy and Communication Theory** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Strategic Family Therapy and Communication Theory** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Strategic Family Therapy and Communication Theory**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a

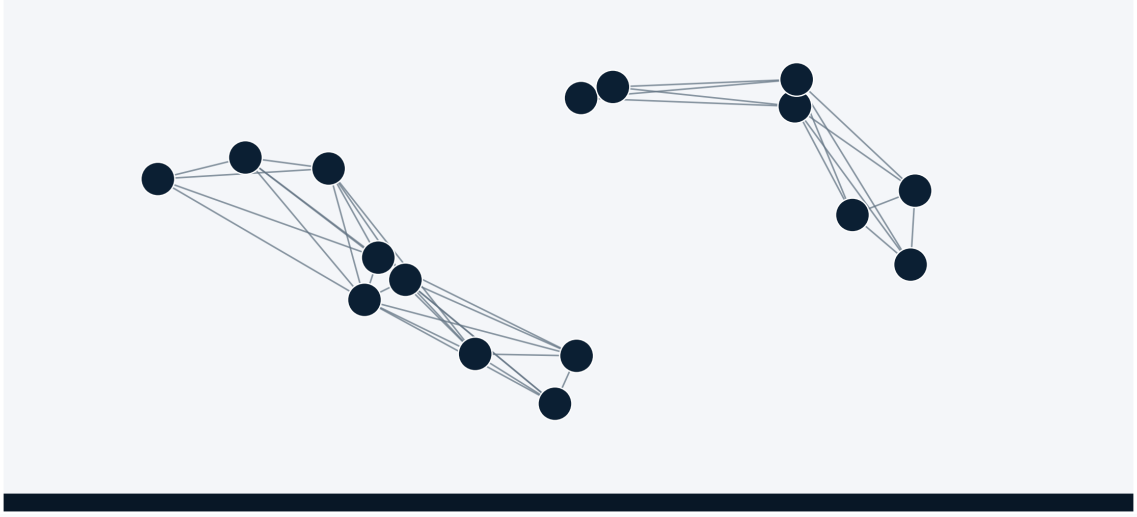
discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Strategic Family Therapy and Communication Theory to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 33

# Family Therapy with Parents and Adolescents

Developmental and systemic consultation with families in conflict.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain why the parent–adolescent relationship is the central developmental relationship of the second decade of life, and why the work with the parents is the essential work.
- Describe the developmental tasks of the adolescent years and the parental tasks that correspond to them.
- Explain the parent training literature and the behavioural analysis of the parent–adolescent interaction cycle.
- Describe the mainstream and conflict patterns of parent–adolescent communication and the clinical significance of each.
- Apply the principles of developmental and systemic consultation to a school or clinic case involving a young person and their parents.
- Design a parent-focused intervention for a case of adolescent non-compliance or conflict, including the components and the sequencing.
- Critically evaluate the cultural and ethical dimensions of work with parents in the adolescent years, including the question of whose problem the work addresses.

## Chapter Overview

This chapter is the practical centre of Part V, and it is the chapter in which the preceding three theories are brought together around a single clinical situation: a young person and their parents in conflict.

The argument of the chapter is simple and it is the most important thing in this part of the book. **When a young person and their parents are in conflict, the parents are the more tractable target.** This is not a value judgement about who is more important. It is a statement about what a practitioner can actually work with. The parents are adults, they are available, they are generally motivated by the referral, and the relationship between them and the young person is the mechanism through which everything else operates. A young person who is caught in the middle of a conflict between two adults cannot think, cannot develop, and cannot use the therapy; the adults can.

The developmental literature makes the same point from the other direction. The adolescent's task is a renegotiation of the relationship with the parents, and the renegotiation is a *developmental* task that can only be completed if the parents make the renegotiation possible. A young person cannot unilaterally leave the child subsystem; the parents hold the key to the door and, if the door stays locked, the young person's options are to force it, to break it, or to give up and stay.

The chapter therefore covers: the developmental rationale; the parent training literature and the behavioural analysis of the escalation cycle; the communication patterns and their clinical significance; the practical structure of a parent-focused intervention; the consultation relationship with the school; and the ethical and cultural analysis, which is substantial because the work with parents raises the question of whose difficulty is being treated.

## 1. The Developmental Rationale

## 1.1 The second decade

The period from approximately ten to twenty is the period in which the young person becomes a person rather than a child, and the mechanism is the renegotiation of the relationship with the parents. This is a period of enormous change in the young person's cognition, in which the ability to think hypothetically and to reason about other people's states develops rapidly, and it is a period in which the peer world becomes the primary arena of development.

The developmental literature gives four accountabilities to the parents during this period, and each is a task that can be observed and worked on.

**To provide a base of secure attachment that the adolescent can gradually relinquish.** The adolescent needs a secure base to move out from, and the secure base is not a helicopter parent; it is the reliable, non-intrusive availability of an adult who is not frightened of the adolescent's development.

**To maintain the parental authority that the adolescent needs.** The authority question is the one that generates most of the conflict in this period, and the adolescent's developmental task is to test the limits of the parental authority, and the parents' task is to hold the limit without needing to win. A parent who cannot hold a limit without a fight will lose the developmental negotiation.

**To provide a structure that is firm enough to be safe and flexible enough to grow into.** The adolescent needs a structure that increases in responsibility as the capacity increases, and a structure that does not increase in responsibility produces a young person who cannot manage, and a structure that increases too fast produces a young person who cannot cope.

**To maintain a relationship with the adolescent that is not the same as the relationship with the child.** The parents have to be able to relate to a person who is not the child they had, and the parental task of mourning the child is routinely skipped, and the young person experiences the mourning as rejection.

## 1.2 The mismatch and the symptom

The clinical presentation in this period is very often a **structural mismatch**: the parents are operating a pre-adolescent structure over an adolescent child, and the symptom is the report of the mismatch. The report is in the language of behaviour — the arguments, the slammed doors, the refusal, the secretiveness, the risk-taking, the disappearing — because the young person has no other channel through which to say "the rules here are for a younger person than I am".

This is the diagnosis that the structural model of Chapter 31 makes most precisely, and it is very frequently the correct one. The clinical test is simple and a student should be able to apply it: **compare the demands and the expectations the parents are making with the developmental capacity of the specific young person.** The demands for a fourteen-year-old's contribution to the household, the expectations about curfews, about the use of the phone, about disclosure, about the treatment of the parents' requests, about the sharing of information — these are the variables.

## 1.3 The task of the practitioner

The practitioner's task in this period has three parts, and they are in a definite order.

**First, to establish what the parents want.** Not what they say they want in the referral, which is usually the symptom's removal, but what they want for the young person's life. Almost every parent in this period wants something they cannot say, because saying it would be to acknowledge that the family is not in the state

they have described. A mother who refers her fifteen-year-old for "attitude" and who actually wants a relationship in which her daughter tells her about her life is a mother who can be worked with, and a mother who wants the daughter to obey is a mother who can be worked with differently.

**Second, to help the parents make the changes that the developmental situation requires.** This is the work, and it is the work with the adults.

**Third, to work with the young person** on the developmental tasks that the improved family environment makes possible.

# 2. The Parent–Adolescent Interaction Cycle

## 2.1 The behavioural analysis of conflict

The parent training literature produced one of the most thoroughly validated findings in the child behavioural literature: the **escalation cycle** in which oppositional and antagonistic behaviour between parents and children is maintained by a predictable sequence of responses, and the sequence is not the one the parents intend.

The cycle runs as follows. The parent makes a request. The child refuses. The parent repeats the request, with escalating insistence, with warnings, with increased volume, and often with the addition of a threat. The child escalates in response. The parent then backs down, either explicitly, or by changing the subject, or by shouting and then leaving, or by threatening a consequence that does not arrive. The child's escalation has produced the parent's withdrawal, and the withdrawal is the reinforcement.

The sequence has three features that a practitioner should be able to name.

**The escalation is a communication.** It is a bid for attention, for a change of activity, for a delay, for help, or for the removal of the demand, and it is the only bid available if the polite bid has been ignored.

**The parent's escalation is the trigger for the child's.** A parent who increases the volume produces a child who increases the volume, and the increase is not a character trait; it is a reaction to a reaction.

**The parent's backing down is the reinforcement.** The behaviour that ends the battle is the behaviour that is rewarded by the end of the battle, and the reward is the removal of the demand, which is the most reliable and most reinforcing consequence available in the situation.

## 2.2 The parent training evidence

The parent training literature is the strongest evidence in the field for the claim that parental behaviour is a tractable target, and the findings are consistent.

**Parent training has a robust effect on oppositional behaviour and on conduct problems**, and the Cochrane reviews report substantial and durable effects, and the effects are maintained at follow-up.

**The improvements are broad.** Parent training produces improvements in the child's oppositional behaviour, in the parent's distress, in sibling conflict, in the family's functioning, and in some studies in the child's academic performance.

**Parent training works best when the parent's behaviour changes and when the family is engaged with the work**, and the fidelity of the parent's use of the techniques determines the outcome.

**Parent training works less well with young people who are severely antisocial**, and the pattern of the research is that the interventions work better with younger children and with less entrenched patterns.

**The sessions with the parents are the sessions.** Almost all parent training programmes involve the parents alone for the core of the work, and the child is not in the room.

### 2.3 The skills parent training teaches

The core content of the parent training that has the best evidence is short and teachable.

**Positive reinforcement and attention.** The parent learns to attend to the desired behaviour promptly, specifically, and with a description of what the child did, and to increase the attention to the desired behaviour relative to the undesired.

**Ignoring and planned ignoring.** The parent learns to withdraw attention from the undesired behaviour, genuinely, without a verbal signal, and the parent learns that the withdrawal is temporary and that the behaviour will get worse before it gets better.

**Time-out from reinforcement** and **response cost**, taught with the specific rules that make them safe and effective.

**Consistency between the adults**, which is the most commonly neglected and most necessary element, and which requires the practitioner to work with every caregiver who has significant contact with the young person.

**The skill of talking rather than shouting**, taught through modelling, role-play, and behavioural rehearsal.

**The "catch them being good" practice**, which is a specific, concrete, and effective technique: the parent is instructed to notice, describe, and respond to every instance of the desired behaviour, and to record them, and the recording is what makes the noticing systematic.

### 2.4 A caution about the behavioural model of parenting

The parent training literature is the strongest evidence base in the field, and it is also the literature with the strongest cultural and moral assumptions, and a student should be able to state the concern.

The model treats the parent's attention as a resource to be allocated according to a set of technical rules, and it treats the child's behaviour as a set of responses to be selected. Two consequences follow. First, the young person is understood as a set of behaviours to be managed rather than as a developing person with a point of view, and a practitioner who works exclusively in this frame will produce a compliant young person and a distressed relationship. Second, the model sits uncomfortably with a body of evidence and ethical argument that the consistent use of reinforcement and the withdrawal of attention is a form of conditioning that is appropriate for a behaviour and inappropriate for a relationship.

The resolution is the one that the evidence itself supports: parent training works when it is used to increase the *quality* of the relationship, not to increase the *compliance* of the child. The techniques that improve the relationship — attending to what the child is doing well, describing it specifically, being consistent, being calm — are the same techniques. The difference is in what the practitioner is trying to achieve with them, and the difference is observable in the outcome.

## 3. Communication Between Parents and Adolescents

## 3.1 The mainstream and the conflict pattern

The developmental literature on parent–adolescent communication identifies a **mainstream pattern** of good communication and a **conflict pattern** of poor communication, and the characteristics of each are the basis of the assessment.

The **mainstream** is characterised by the parents listening without interruption; by the parents being accepting of the young person's feelings even when they disapprove of the behaviour; by the parents acknowledging the young person's point of view as a legitimate one; by the parents being reasonably consistent; by the young person disclosing; by the parents being involved in the young person's world; and by the parents showing a mild sense of humour.

The **conflict pattern** is characterised by the parents' frequent criticism; by the parents' hostility and their belittling of the young person; by the parents' physical punishment; by the parents' inconsistent discipline; by the young person's withholding of information; by the young person's refusal to talk; by the young person's escape from the family; and by the young person's physical and verbal aggression outside the home.

The patterns are almost perfectly correlated with the outcomes, and the correlation is large. A student should know the list and should be able to use it as an assessment instrument in the first session, because the two patterns are readily observable in a single family session.

## 3.2 The disclosure process

The literature on adolescent disclosure identifies a developmental sequence with direct clinical implications.

**The early childhood phase** (approximately six to twelve) is one of high parental control and high disclosure, in which the parents determine what is disclosed and the young person does not select.

**The early adolescence phase** (approximately twelve to fifteen) is one of decreasing parental control and a shift towards peer orientation, in which the young person's disclosure to the parents declines and the parental knowledge of the young person's world declines with it. The developmental task of this phase is described as **detachment from parental control**, and it is a normal and necessary stage.

**The middle adolescence phase** (approximately fifteen to seventeen) is one of increasing peer orientation and of the development of an **individualised system of beliefs, values, and perceptions** that is often incompatible with the parents'. The developmental task is the development of an independent moral and value system, and the young person needs to have arguments in order to do it, and the arguing is the work, not the failure.

**The later adolescence phase** (approximately seventeen to twenty) is one of a gradual re-establishment of a relationship with the parents that is more equal and more companionate, and the relationship that eventually forms is the one in which the young person can disclose.

Two clinical conclusions follow, and they are important.

**The decline in disclosure during early adolescence is normal and is not a problem to be solved.** A practitioner who treats the young person's refusal to tell the parents about their weekend as a symptom is doing something harmful, and a parent who experiences it as a symptom will respond with intrusion, and the intrusion produces more withdrawal. The developmental task is detachment from parental control, and the parent's task is to allow it.

**The young person's arguments in the middle adolescent years are part of the work.** The developmental literature is unusually clear that the "who owns me" arguments are necessary for the development of an individualised value system, and that a parent who wins these arguments wins them at the cost of the young person's development. The parent's task is to survive them, and to set the limits on the *form* of the argument while not requiring the young person to abandon the content.

### 3.3 The three common conflicts

Three conflicts recur in clinical work, and each has a different clinical character.

**The autonomy conflict.** The young person is asserting control over choices — clothing, friends, music, the use of the phone, the choice of clothes, the choice of music, the choice of friends, the choice of what to wear, the choice of where to go, the choice of which music to listen to, the choice of which friends to see. The underlying developmental issue is the differentiation of self, and the parental contribution is usually the overreaction. A parent who treats a preference about clothing as a challenge to authority produces a fight in which neither the clothing nor the authority is really the point.

**The sexuality conflict.** This conflict, which arises in the context of the family's boundaries, the young person's emerging sexuality, the family's cultural and religious values, the parents' own relationship with their own bodies, and the question of privacy, is unusually charged and it requires cultural competence rather than technical skill.

**The achievement conflict.** The young person's performance in school is the site of the parents' own histories, their own educational experience, their own social class, and their own comparative standing. This conflict is frequently a conflict between the young person and the parent's projected self, and it is the conflict in which the family therapy work is most clearly indicated, because the parents' investment in the young person's achievement is not the young person's own.

## 4. Further Reading for This Chapter

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# 5. The Practice of Parent–Adolescent Work

## 5.1 The first session: the parents alone

The first session is with the parents, and it is the session in which the work is determined, and it has a specific structure.

**The referral and the actual request.** The practitioner asks what the parents hope will be different, and then asks the question that reframes the session: "And what do you want for [the young person's name] in three years' time? Not what you want them to stop doing — what you want their life to look like." The two questions usually produce different answers, and the second is the one the work is built on.

**The developmental timeline.** The practitioner asks the parents to describe their child's development, from the earliest years they can remember, and the narrative does a great deal of the work. Parents who describe a child who was always difficult are describing a child they have never had a relationship with; parents who describe a child who was easy until a particular point are identifying the point at which the difficulty began, and the point is often the point at which the developmental task changed and the parental structure did not.

**The pattern.** The practitioner asks about a specific recent instance, and gets the sequence, using the material of Chapter 32. The sequence is the most useful single artefact of the assessment, because it is specific, and because it identifies the change points.

**The escalation.** The practitioner asks the parents to describe what they do when the young person refuses. The answer is the escalation, and most parents have not heard themselves describe it.

**The parents' own lives.** The practitioner asks what has been happening in the parents' own lives over the last year. A new job, an illness, a bereavement, a debt, a pregnancy, a separation, a new baby, a move, a caring responsibility for an older relative. The list of things that can precipitate a family crisis in this period is short, and the practitioner should have it in mind, and the answer is frequently that something has

happened.

## 5.2 The five components of a parent-focused intervention

**Component one: the psychoeducation.** The parents are told what the developmental task is and what the developmental timetable looks like, and the specific points about the decline in disclosure, the arguing, and the detachment from parental control are explained. The explanation is not to make the parents feel guilty; it is to remove the interpretation that the young person's behaviour is a rejection of them, which is the interpretation that generates the escalation.

**Component two: the reconstruction of the sequence.** The parents and the practitioner map the escalation cycle together, on paper, in the session. The map is the therapeutic instrument, because the parents can see the cycle only once it is drawn. The practitioner should expect the moment of recognition, and it is often the most important moment in the whole course of work, and it is also the moment at which a parent becomes hopeful, because the cycle implies that it can be interrupted.

**Component three: the skills.** The parent training content is taught: positive reinforcement, attention to the desired behaviour, planned ignoring, time-out from reinforcement, consistency, and the "catch them being good" practice. The skills are taught by rehearsal, not by discussion, and the rehearsal is the reason parent training works better than information about parenting.

**Component four: the renegotiated structure.** The specific changes to the rules: the curfews, the phone, the privacy, the contribution to the household, the disclosure, the consequences. The changes should be written down, agreed, and dated, and they should include an explicit statement of what the young person gets as well as what is required. A set of rules that only restricts is a set of rules that will be breached.

**Component five: the relationship work.** The most important and the most neglected component. The parents and the young person need a relationship that is not the one in which the conflict occurs, and the work is on the *quality of the connection* rather than on the *management of the behaviour*: the specific daily contact, the specific appreciation, the specific time, the specific activity. A father who has a Saturday morning hour with his fifteen-year-old son and does something the son chooses has built something that the token of a good week of compliance cannot buy.

## 5.3 Sequencing: the parents first

The sequencing follows from the developmental reasoning and should be defended to the referrer if the referral expects to see the young person in the first week.

**Weeks 1–4: the parents.** The psychoeducation, the sequence, the skills, the renegotiated structure. The young person is seen briefly and individually in parallel, for the alliance and for the developmental work.

**Weeks 5–8: the parents and the young person together.** The renegotiated rules are made explicit, the young person is given a real role in the negotiation, and the practical arrangements are settled.

**Weeks 9 onward: the work with the young person**, which can now proceed because the environment has changed.

The strongest practical argument for this sequence is empirical: a young person who is asked to change while the adults who control the environment are not changing will conclude that the adults are not serious, and the conclusion will be correct, and the young person's cooperation will be withdrawn. A young person who is asked to change after the adults have changed will conclude that the change is real, and the

conclusion will also be correct, and the young person's cooperation will be available.

## 5.4 When the parents will not work

Sometimes the parents will not or cannot do the work, and the practitioner needs a plan.

**The parents are the problem.** In a minority of cases the young person is the least disturbed member of the family, and the work is with the parents' own difficulties. The practitioner should be able to identify this situation, and the referral should be redirected rather than the work being forced.

**The parents cannot agree.** A common situation is that the two parents want different things. The work is then directed at the *disagreement*, and a common and effective intervention is to help the parents reach an agreement about the rules as a preliminary to anything else. Parents who cannot agree on a curfew cannot implement a curfew.

**The parents are overwhelmed.** A family with three jobs, no transport, and a young person with additional needs cannot undertake a skills programme. The response is to reduce the intervention to its minimum viable form: one specific rule, changed, consistently applied.

**The young person refuses to participate.** The work continues with the parents, and the young person's refusal is treated as information, and the practitioner maintains a separate channel with the young person.

## 5.5 Consultation with the school

Almost every case of adolescent conflict involves the school, and the school is usually where the young person's difficulties are most visible and where the parents' changes are least likely to be supported. The consultation work has four elements.

**The school's account of the behaviour.** The behaviour in the corridor, in the classroom, and in the dining hall may look entirely different from the behaviour at home, and the school is a different system with different contingencies.

**The alignment of the responses.** If the parents have made a new rule and the school has a different one, the young person will be confused and will be given the opportunity to play the two against each other. The alignment should be explicit and should be written down and shared.

**The information flow.** The school is a source of information about the young person, and the practitioner must decide what is shared with the school and what is not, and the young person must know, and the consent requirement is absolute. In most cases, the young person's grades, the school reports, and the practitioner's observation are shared with the parents and not with the school, unless the young person agrees.

**The teacher's needs.** The teacher is dealing with the behaviour in the classroom, and the teacher needs something concrete. A specific plan for the classroom — a specified response to the disruption, a specified alternative the young person can use, a specified signal for help — is worth more than a general reassurance.

**Developmental Tasks and the Parent-Adolescent Tension**

| Developmental task of the adolescent | Typical parental worry | Constructive counselor stance |
|---|---|---|
| Autonomy and independence | Loss of control, loneliness, risk | Separate the person from the conflict |
| Identity formation | Loss of influence over who the child becomes | Support exploration without fixating an outcome |
| Peer and romantic relationships | Values, loyalty, reputation | Hold the boundaries that matter, negotiate the rest |
| Risk and sensation seeking | Physical danger, moral failure | Distinguish safety from disapproval |
| Sexual and romantic development | Abandonment, vulnerability | Provide accurate information without intrusion |

*Most family conflict with adolescents is a developmental artefact rather than a pathology.*

**Figure 33.22** The parent–adolescent relationship shown in three domains, with the parental tasks, the adolescent tasks, and the points of conflict identified for each developmental phase.

### 5.6 Working with the young person directly

The work with the young person is not only a phase that comes later. Several interventions can be delivered directly.

**The developmental work.** The practitioner works with the young person on the specific tasks of the period: the separation from the parental system, the development of an individualised value system, the development of the peer world, the toleration of the inevitable arguments. The work is developmental and it is not reducible to a coping skill.

**The relationship with the young person as a buffer.** Even where the parents cannot change, the practitioner can be the adult in the young person's life who is reliable, non-intrusive, and available, and the young person's experience of being able to trust an adult is itself a protective factor and a developmental achievement.

**The work on the specific difficulty.** Where the young person's difficulty is a symptom — the anxiety, the depression, the eating, the substance use — the specific treatment is delivered in parallel, and Chapter 38 and Chapter 47 address the complex cases.

**The mediation.** Where the parents cannot work together, the practitioner can work with the young person and one parent on the relationship, and the work can produce a change in the dyad that the family system then accommodates.

## 6. Case Illustration: The Young Man Who Had Been Given Ultimatums

**Presentation.** Samuel is fifteen and lives with his mother and his younger sister. He is referred by the school, which is concerned about his behaviour in lessons. His mother attends the first appointment and is, in the practitioner's word, "furious and frightened in about equal measure". The referral describes a young man who is argumentative, who does not do his homework, who is "rude" to his mother, and who has

recently been excluded twice.

**The first session with the mother.** The practitioner begins with the question that reframes the session: "What do you want for Samuel in three years' time?" The mother says, without hesitation: "I want him to go to university. I want him to have what I didn't have." The practitioner asks what she wants him to stop doing, and the mother says: "The arguing. The rudeness. The not doing his work."

The two answers are in different places and the difference is the whole case. The mother's stated goal is a future, and it is a future she has a strong investment in, and it is a future in which Samuel is the vehicle for the repair of her own history. The referral question is about the present, and it is about compliance.

**The sequence.** The practitioner asks the mother to describe a recent argument in detail. What emerges is a highly patterned exchange. Samuel comes in at nine; his mother asks him about his day; he says "fine"; she asks again; he says "I said fine"; she says "don't talk to me like that"; he says "what do you want me to say, mum"; she says "the truth"; he says "there's nothing to say"; she raises her voice and says "don't give me that attitude, I've had enough, you're going to sit at that table until you've done your work"; he goes up to his room; she calls up after five minutes; he does not answer; she says "fine, be like that"; and the evening ends with both of them certain that the other is the problem.

The practitioner maps it on paper, with the mother, and the mother's face changes when she sees it. The sequence is recognisable: the repeated question after "fine", the "don't talk to me like that", the demand that he produce an emotion he does not have, the ultimatum about the table, the five-minute silence, and the withdrawal. And then the practitioner asks the critical question: "What happens if he does sit at the table?" The mother says: "He doesn't. He just talks." The practitioner asks: "So what does he get by not sitting at the table?" The mother thinks and then says, with some surprise: "He gets to be upstairs."

**The formulation.** Samuel's refusal is maintained by escape. He is not in conflict with his mother; he is a young man who has discovered that a specific sequence of exchange delivers him to his room, and the room is where he wants to be, and it is the only thing in his day that is reliably his. The mother's ultimatums are the problem, and they are also the solution — they are the family's only route to the room, and they have taught Samuel that the negotiation is the way to get there.

**The intervention with the mother.** Four elements.

*Element one: the developmental frame.* The practitioner explains the mid-adolescent phase: the arguing, the testing, the increasing need for an individualised value system, the decline in disclosure, the detachment from parental control. The explanation is delivered with the developmental literature, and it is delivered as information rather than as correction, and the practitioner is careful to say that the arguing is necessary and is not going to stop.

*Element two: the restriction.* The practitioner gives the mother a specific instruction: "For the next two weeks, you are not going to ask him a second time about anything. If he says 'fine', that is the end of the conversation about his day. If he says nothing, that is also the end. The only exception is the question about the work, and the work question is going to be a different conversation, and I will show you how."

The reasoning is given explicitly: the repeated question is the hook, and the young man is not resisting the question, he is resisting the repetition, and the repetition is what gives him the opening.

*Element three: the work question.* The practitioner spends twenty minutes with the mother on how to ask about the homework, and the difference is technical but decisive. "How's the homework?" produces "fine" and then a confrontation. "What did you get through on the maths?" produces a description. The second

question cannot be answered with "fine" and it does not require the young person to produce a feeling, and it opens a conversation about a specific thing.

*Element four: the negotiation with Samuel.* The practitioner meets Samuel alone and explains what is happening, in terms that do not disclose his mother's account of the sequence: "Your mum and I have been looking at what happens when she asks you about your day. You've probably noticed that when she asks twice, things get worse. She's going to try asking once and then leaving it. I wanted you to know it's happening so it doesn't feel like a trick." Samuel considers this and says: "She'll forget." The practitioner says: "Probably. And then you can tell her, and we'll do it again."

**The outcome.** The change is visible in three weeks. The arguments stop; what replaces them is silence, and the practitioner explains to the mother that silence is a good outcome here, because the silence is the absence of a fight rather than the presence of a wall. Samuel comes down of his own accord more often, and the work gets done more often, and the two of them have a relationship that is not warm but is not combative. The school reports an improvement in the lessons, though less dramatic than the mother hoped, and the practitioner is careful to say why: the school is a different system and the changes have to be made there too, and the consultation with the form tutor is what produces them.

**The interpretation.** The case illustrates the whole of the chapter. The referral was about compliance. The problem was the escape contingency that the mother's ultimatums had created. The work was with the mother, because the mother was the tractable target, and because the developmental argument was the thing that changed her behaviour. And the practitioner did the thing that is hardest in this work, which was to tell the mother that the arguing would not stop, and that the goal was a different relationship and not the elimination of the disagreement.

# 7. Evidence and Research Support

## 7.1 The status of the evidence

The evidence for parent-focused work with adolescents is the strongest in the field, and a student should be able to summarise it precisely.

**Parent training has a robust effect on oppositional behaviour and conduct disorder**, and the Cochrane reviews report substantial effects, maintained at follow-up, and the effects are larger for younger children than for adolescents.

**Parent training improves the parent's own symptoms of distress**, and the parental depression and the parental stress are among the better-established effects, which is one of the strongest arguments for the approach, because the parent's distress is what maintains the escalation.

**Parent training is effective in school-based formats**, and the evidence for the school-based delivery of parent training is good, and the school-based formats are more accessible to families who cannot attend clinic appointments, and the evidence for the telephone and the online formats has grown.

**Behavioural family therapy, which combines parent training with the teaching of problem-solving and communication skills, has a strong evidence base**, and the Cochrane reviews of behavioural family therapy for conduct problems report large effects.

**Family-based treatment of adolescent substance use has good support**, and the family-based approaches consistently outperform the individual approaches in this population.

**Parent-focused interventions improve the young person's functioning as well as the behaviour**, and this is one of the more important findings, because the changes in family functioning are what make the behaviour change durable.

**The effects are smaller for adolescents than for younger children**, and the pattern of the research is that the more entrenched the pattern and the older the young person, the smaller the effect.

### 7.2 The limitations of the evidence

**The studies are largely North American and European**, and the cultural specificity of the parenting constructs in them is significant.

**The samples are selected**, and the families who complete parent training are more engaged and less disadvantaged than the families who are referred.

**The measures are largely behavioural and parental-report**, and the young person's own account of the change is less often measured.

**The long-term follow-up is limited**, and the question of whether the effects are maintained into the adult years is largely unanswered.

**The studies rarely examine the mechanism**, and it remains difficult to specify which components of the parent training package are responsible for the effect.

## 8. Strengths and Limitations of Parent-Focused Work

### 8.1 Strengths

**It works.** This is the strongest empirical claim that can be made for any intervention with adolescents and their families, and it is unusual in this field.

**It addresses the adults who control the environment.** A young person cannot change a household; a parent can.

**It is deliverable in schools and in primary care**, and it is deliverable by non-specialists with supervision, and the evidence for the supervised delivery is good.

**It is short.** Parent training is typically between eight and sixteen sessions, and brevity is a clinical advantage with a population that does not stay.

**It improves the parents**, and the parents' improvement is a durable benefit even if the young person's symptoms recur.

**It does not require the young person to be motivated.** This is a significant advantage: a young person who is not engaged can still benefit from a change in the environment.

### 8.2 Limitations

**It can be used in ways that are not therapeutic.** The techniques of parent training can be used to produce compliance in a young person whose problem is not compliance, and the field has a genuine responsibility here.

**It can locate the problem in the parent when the problem is elsewhere**, and this is the principal risk of the approach.

**It says little about the young person's own experience**, and a practitioner working exclusively in the parent frame will produce a family that functions and a young person who is not seen.

**The effects on adolescents are smaller than on younger children**, and the hardest cases are the ones that most need it.

**It assumes a stable, cooperative, two-parent family**, and a substantial proportion of the families in contemporary services are neither stable nor cooperative nor two-parent.

# 9. Cultural and Ethical Considerations

## 9.1 Whose problem is being treated?

This is the central ethical question of the chapter, and it should be asked in every case.

When a young person is referred for conflict with their parents, the referral frames the young person as the problem. The ethical work is to establish whether the young person is the problem, the parents are, the family system is, or the circumstances are. Four different interventions follow, and a practitioner who does not make the distinction will deliver the wrong one.

**The young person is the problem.** The conflict is a symptom of the young person's difficulties, and the parents need support in managing a difficult developmental period, and the work with the young person is the primary work.

**The parents are the problem.** The young person is the least disturbed member of the family, and the work is with the parents, and the young person's needs are secondary to the parents' in this particular case.

**The family system is the problem.** The conflict is the family's; the young person is the site.

**The circumstances are the problem.** The conflict is a rational response to conditions — poverty, a disability, a housing crisis, a shift pattern, a family member's illness, a school that is unsafe. A parent who is working two jobs and is raising a disabled child alone and is being told by the school that her son is "difficult" is not a parent with a parenting problem.

A practitioner who does not ask the fourth question will systematically misdiagnose the families who most need help.

## 9.2 Cultural considerations

**The parenting norms of the practitioner are not the parenting norms of the family.** The practices that the parent training literature treats as effective — the daily attention, the specific praise, the consistent response, the clear rules, the negotiated decision-making — are specific to a particular cultural and historical context, and many families use other systems that work.

**Some of the most "effective" practices are the most damaging across cultures.** A parent who has been taught to withdraw attention as a response to a child's bid may, in a context where the child's bid is a call for connection in a context of scarcity, be withdrawing the very thing the child needs. The practitioner must know the family's context before implementing the ignoring techniques.

**Authoritative versus authoritarian.** The parenting literature's preferred style is authoritative — high in warmth and high in structure — and the style is a culturally specific form, and a family that is authoritative in its own terms may be described by an observer in the field's terms as authoritarian.

**The father's role.** The evidence base and the practice of parent training have been developed disproportionately with mothers, and the involvement of fathers is consistently lower than it should be, and a programme that does not include the father in a family where the father is present and available reproduces a structural problem.

**Language and the training materials.** The parent training materials are written in a particular register and a particular idiom, and a family whose first language is different will need the concepts translated and the materials adapted, and a practitioner who hands over a translated leaflet has not adapted the intervention.

### 9.3 Ethical requirements

**Informed consent from the parents and, where the young person is engaged, their informed participation.** A parent who consents to a skills programme under the impression that it will make their child comply has been misled.

**The young person's right to be heard.** Even where the work is with the parents, the young person's account must be sought, and it must be sought by someone the young person trusts.

**Confidentiality.** The boundaries between the individual work with the young person and the family work must be explicit and must be stated to everyone, and the practitioner must be able to state what will be shared with the parents and what will not.

**The prohibition on the use of the work to control.** A practitioner should decline a referral whose stated purpose is to obtain compliance, and should say why, and should offer the alternative.

**Proportionality.** The intervention should be proportionate to the difficulty, and the practitioner should resist the temptation to impose a full programme on a family with a small and specific difficulty.

**Equity of access.** Parent training delivered in a clinic to families who can attend is a service that will systematically under-reach. The school-based, the telephone, and the online formats exist because of this, and a practitioner in a service with a poor attendance record should be actively looking for them.

## 10. Key Terms

**Key Terms at a Glance**

- **Parent–adolescent conflict** — The friction between the parents and the adolescent in the second decade, which is a normal feature of the developmental renegotiation and becomes a clinical problem when it is escalating and the relationship is deteriorating.
- **The developmental task of the second decade** — The renegotiation of the relationship with the parents through which the young person becomes a person rather than a child.
- **The four parent accountabilities** — To provide a secure base that can be relinquished, to maintain the authority the adolescent needs, to provide a structure that is firm and flexible, and to maintain a relationship with the young person that is not the one they had as a child.
- **The parental task of mourning the child** — The work of relating to the young person the adolescent has become, which is routinely skipped and is experienced by the young person as rejection.
- **Escalation cycle** — The patterned sequence in which the parent's escalation produces the child's escalation and the parent's backing down then reinforces the child's.
- **The three features of the escalation cycle** — The escalation is a communication, the parent's escalation triggers the child's, and the parent's backing down is the reinforcement.
- **Parent training** — A structured programme of behavioural and communication skills for the parents, delivered in groups or individually, that has the strongest evidence base of any intervention with families.
- **Positive reinforcement and attention** — The parent skills of attending promptly and specifically to the desired behaviour, with a description of what the child did.
- **Planned ignoring** — The complete and temporary withdrawal of attention from a behaviour, taught with the expectation of an initial increase.
- **Time-out from reinforcement** — A specified removal of access to reinforcement contingent on a behaviour, taught with the rules that make it safe and effective.
- **Consistency between the adults** — The agreement of all caregivers who have significant contact with the young person, which is the most commonly neglected and most necessary element of the work.
- **The "catch them being good" practice** — The systematic noticing, describing, and responding to instances of the desired behaviour, with a record.
- **Mainstream communication pattern** — The characteristics of good parent–adolescent communication, and the profile of the families with better outcomes.

### Key Terms at a Glance (continued)

- **Conflict communication pattern** — The characteristics of poor parent–adolescent communication, and the profile of the families with worse outcomes.
- **The four phases of disclosure** — High control and high disclosure in early childhood, decreasing parental control and declining disclosure in early adolescence, peer orientation and the development of an individualised value system in middle adolescence, and the re-establishment of a more equal relationship in later adolescence.
- **Detachment from parental control** — The early adolescent developmental task, and the reason the decline in disclosure is normal rather than a symptom.
- **The individualised value system** — The middle adolescent developmental task, and the reason the arguments are part of the work rather than evidence of its failure.
- **The three common conflicts** — The autonomy conflict, the sexuality conflict, and the achievement conflict, each with a different clinical character.
- **The achievement conflict** — The conflict over the young person's school performance, which is frequently a conflict with the parent's projected self.
- **Reformulation of the request** — The clinical move of distinguishing the parents' request for the removal of a behaviour from their wish for the young person's life.
- **The reconstruction of the sequence** — The mapping of the escalation cycle on paper with the parents, which is the therapeutic instrument of the parent-focused work.
- **The renegotiated structure** — The specific, written, dated set of changes to the household rules, including what the young person receives as well as what is required.
- **The relationship work** — The component directed at the quality of the connection between the young person and the parents, as distinct from the management of the behaviour.
- **Sequencing the parents first** — The clinical sequence in which the parents' work precedes the family's joint work, and the empirical basis for it.
- **The minimum viable intervention** — The reduced form of parent work, consisting of one specific rule changed and applied consistently, for families who cannot undertake a full programme.
- **The parent's disconfirmation** — The repeated question, the demand for a feeling, and the ultimatum, and their role in producing the confrontation.
- **The "what did you get through on the maths" question** — A specific formulation that cannot be answered with "fine" and that opens a conversation about a particular thing.

**Key Terms at a Glance (continued)**

- **Consultation alignment** — The requirement that the rules and responses in the school match those in the home, and the written record of the alignment.
- **The school's own contingencies** — The different reinforcement structure of the school setting, which requires its own functional analysis and its own plan.
- **Whose problem is being treated** — The ethical question of whether the young person, the parents, the family system, or the circumstances constitute the problem, and the four different interventions that follow.
- **The circumstances as the problem** — The category in which a young person's behaviour is a rational response to material conditions, and the failure to consider it that misdiagnoses the families most in need.

## 11. Chapter Summary

This chapter brought the three preceding theories together around the central clinical situation of this part of the book, and its governing argument is that when a young person and their parents are in conflict, the parents are the more tractable target. The argument is developmental rather than moral: the adolescent's task is a renegotiation of the relationship with the parents, the young person cannot unilaterally leave the child subsystem, and the adults hold the door.

The developmental rationale was set out with the four parent accountabilities, and the clinical presentation was described as a structural mismatch — the parents operating a pre-adolescent structure over an adolescent child — with a simple clinical test: compare the demands with the developmental capacity of the specific young person. The three-part task of the practitioner was described, and the most important element was the clarification of what the parents actually want, which is almost never the removal of the behaviour.

The behavioural analysis of the escalation cycle was described in detail, with its three features, and the parent training evidence was summarised, including the breadth of the effects and the finding that the sessions with the parents are the sessions. The core skills were described, and a substantial caution was given: the model treats the parent's attention as a technical resource, and a practitioner who works exclusively in that frame will produce a compliant young person and a distressed relationship. The resolution proposed was that parent training works when it is used to increase the quality of the relationship rather than the compliance of the child, and that the techniques are the same.

The communication material was described with the mainstream and conflict patterns as an assessment instrument usable in a single family session, and the four phases of disclosure with the two clinical conclusions that follow: the decline in disclosure in early adolescence is normal and is not a symptom to be treated, and the arguments of the middle adolescent years are part of the developmental work rather than its failure. The three common conflicts were described, and the achievement conflict was identified as the one in which the family therapy work is most clearly indicated.

The practice section gave the structure of the first session with the parents, the five components of a parent-focused intervention, the case for sequencing the parents first with its empirical basis, the four

reasons the parents might not be able to do the work, the four elements of school consultation, and the interventions that can be delivered directly to the young person.

The case of Samuel demonstrated the chapter. The referral was about compliance; the sequence revealed an escape-maintained refusal produced by the mother's own repeated questions; the restriction broke the cycle; the work question was re-specified so that it could not be answered with "fine"; and the practitioner told the mother explicitly that the arguing would not stop and that the goal was a different relationship rather than the elimination of the disagreement.

The evidence was summarised with its limits, and the strengths and limitations were set out with the principal risk identified: that the approach will locate the problem in the parent when the problem is elsewhere. The ethical section took up the question of whose problem is being treated, with four categories and the corresponding interventions, and made the point that a practitioner who does not ask the fourth question will systematically misdiagnose the families who most need help. The cultural analysis addressed the specificity of the parenting constructs, the possibility that the "most effective" practices are the most damaging across cultures, the authoritative and authoritarian distinction, the under-involvement of fathers, and the adaptation of the materials. The ethical requirements closed the chapter with the prohibition on the use of the work to obtain compliance and the requirement of equity of access.

## 12. Review and Discussion Questions

**Review and Discussion Questions**

1. State the chapter's central argument and give the developmental reason for it.
2. State the four parent accountabilities in the adolescent years.
3. Explain the parental task of mourning the child, and the consequence of its omission.
4. Define the structural mismatch and state the clinical test for it.
5. State the three-part task of the practitioner and explain the first element.
6. Describe the escalation cycle and state its three features.
7. Summarise the parent training evidence, including the breadth of the effects.
8. State the six core skills of parent training.
9. Explain the caution about the behavioural model of parenting and state the resolution.
10. State the characteristics of the mainstream and the conflict communication patterns.
11. Describe the four phases of the disclosure process.
12. State the two clinical conclusions that follow from the disclosure research, and explain the harm of treating the decline in disclosure as a symptom.
13. Describe the three common conflicts and identify the one in which the family therapy work is most clearly indicated.
14. Describe the structure of the first session with the parents.
15. State the five components of a parent-focused intervention.
16. Explain the case for sequencing the parents first, with the empirical basis.
17. Describe the four situations in which the parents will not or cannot do the work, and the response to each.
18. State the four elements of school consultation.
19. State the interventions that can be delivered directly to the young person.
20. Describe the case of Samuel and state the three features of it that generalise.
21. Summarise the evidence for parent-focused work and state its five limitations.
22. State the six strengths and five limitations of parent-focused work.
23. State the four categories of "whose problem is being treated" and the intervention that follows from each.
24. Explain the four cultural considerations for parent-focused work.
25. State the six ethical requirements for this work.

## 13. Practice Activities

**Practice and Application**

### Activity 33.1 — Mapping the Escalation Cycle (75 minutes, field-based)

Take a real case from placement, or a relationship you are closely involved in, and map the escalation cycle on paper, in the specific terms in which it occurs, with the actual words that are used. Then mark the point at which the cycle would break if one person did something different, and write what that something different would be. Then write the sentence that would introduce that change to the family without it sounding like an accusation. The last step is the one that matters, and most students discover that the change is easy to specify and hard to introduce.

### Activity 33.2 — The Mainstream and Conflict Audit (60 minutes, individual)

Using the mainstream and conflict pattern lists from Section 3.1, rate a family you know on each of the twenty characteristics. Then write the three characteristics on which the family most clearly deviates from the mainstream, and for each, write the specific practice that would move it. The point of the exercise is the discipline of moving from a general impression to a specific behaviour, and of resisting the temptation to work on the twenty characteristics rather than on the three.

### Activity 33.3 — The Three-Year Question (60 minutes, triadic)

One student plays a parent who has referred their fifteen-year-old for "rude and lazy"; one plays the counsellor; one observes. The counsellor's task is to ask the question "What do you want for your daughter in three years' time?" and to follow it wherever the parent goes, without rescuing the parent from their own answer. The observer's task is to note the moment at which the parent's account of the problem changes, and to write down the exact wording at that moment. Almost every group finds that the parent's answer to the three-year question is not about compliance at all.

### Activity 33.4 — The Consultation Map (60 minutes, small group)

In groups of three, take a school-based case and draw a map of the behaviours at home and at school, with the specific antecedents and the specific consequences in each setting. Then identify the three places where the settings are giving contradictory messages to the young person, and write the specific conversation needed with the school. Then identify what the practitioner must ask the young person before the school conversation happens. The third step is the ethical step, and the groups consistently discover that the third step has been omitted in most of the cases they have known.

## 14. Annotated Reading Pathways

**For the developmental rationale.** Read Steinberg (2014) for the authoritative parenting account, Sroufe (1995) for the attachment foundation, and Smetana (1995) for the disclosure research. The disclosure research is the most clinically useful of the three and the most frequently unknown.

**For the intervention.** Read the Oregon model manual and Dishion and McDonough for the intervention science, and read a parent training curriculum in full. The curriculum is more useful than the research literature for practice.

**For the consultation.** Read the school-based family intervention literature, and read the consultation literature on aligning school and home. The alignment problem is the one that most often defeats an otherwise sound plan.

**For the ethics and culture.** Read the evidence on cross-cultural variation in parenting, read the literature on the use of parent training with families who do not share the practitioner's parenting norms, and read the

ethical guidance on the use of behavioural techniques with families.

**For the harder cases.** Read the family-based treatment literature for adolescent substance use and for eating disorders, and read the multisystemic therapy literature. These are the cases in which the parent-focused work needs to be embedded in a larger system, and they are the ones that most often present.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Family Therapy with Parents and Adolescents** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is developmental and systemic consultation with families in conflict. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Leah is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Leah's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Leah explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Leah chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Leah has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Leah can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Family Therapy with Parents and Adolescents

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Family Therapy with Parents and Adolescents** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Family Therapy with Parents and Adolescents** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Family Therapy with Parents and Adolescents**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into

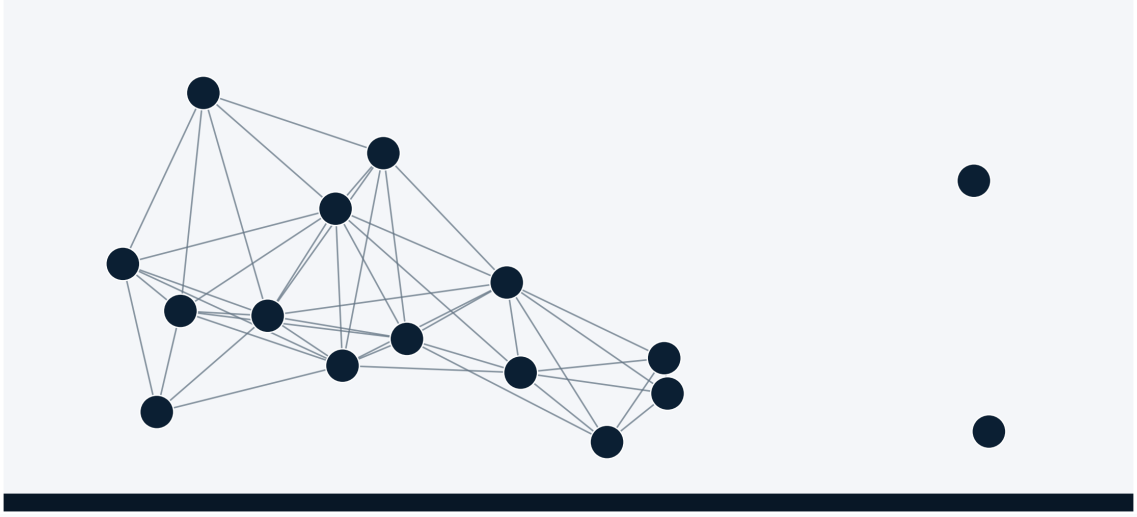
clinical governance.

**Written consolidation.** In 250 words, explain Family Therapy with Parents and Adolescents to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 34

# Attachment Theory and Attachment-Based Therapy

Bowlby, Ainsworth, and the translation of attachment research into treatment.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain Bowlby's account of attachment, including the behavioural control systems, the function of attachment, and the conditions of its formation.
- Describe the research tradition that established the attachment patterns, including the strange situation and the Adult Attachment Interview.
- State the four attachment categories in children and the three in adults, and describe the developmental implications of each.
- Explain the construct of the internal working model and describe its developmental trajectory.
- Describe the principles of attachment-based therapy and its main variants, including the Child and Family Traumatic Stress Intervention and the dyadic developmental psychotherapy.
- Apply the attachment framework to a case involving a young person in a family with a history of loss or maltreatment.
- Critically evaluate the cultural and ethical dimensions of attachment-based work, including the questions of the cultural specificity of the attachment categories and the status of the caregiver's own attachment history.

## Chapter Overview

Attachment theory is the developmental foundation on which much of the practice of this book rests, and it is the theory with the most robust empirical base in the developmental sciences. It is also, in the form most practitioners encounter it, the theory that has been most simplified and most misused.

The theory's central claim is that attachment is a **behavioural system** — a set of evolved, functional, and biologically prepared systems that the infant uses to regulate affect, to explore, and to form a model of the world and of the availability of others. It is not, in the form in which it is often described, a claim about love, and it is not a claim about dependency, and the practitioner who treats it as either will be working with the wrong construct.

The claim that matters most for this book is that the attachment relationship is the organising context of the child's behaviour, and that the internal working model built from that relationship — the expectations about the availability of others, about their trustworthiness, and about the self as worthy or unworthy of care — is carried forward into the child's understanding of other people and of themselves. The adolescent who is unable to trust, who expects to be abandoned, who cannot accept comfort, and who believes that needing anyone is dangerous, is presenting a working model, and the working model was built somewhere.

The theory has also been the source of a serious scientific dispute, and the dispute should be known: the question of whether attachment patterns are categorical or dimensional, the question of the cross-cultural universality of the patterns, the question of the relationship between attachment and the individual-differences measures, and the question of whether the adult attachment interview measures the same construct as the strange situation. The discipline's handling of these disputes is itself a lesson in how a field should treat its own instruments.

# 1. Historical Context

## 1.1 From psychoanalysis to ethology

John Bowlby trained in psychoanalysis and was, for most of his career, in substantial opposition to the psychoanalytic tradition. His central objection was methodological: psychoanalysis proposed a set of developmental stages and a set of intrapsychic mechanisms that were, in his phrase, "unknowable" and therefore untestable, and the claim that the mother was the sole and exclusive object of the infant's attachment was, in his view, supported by evidence that was of a piece with the phrenology of the nineteenth century.

Bowlby's answer was to turn to ethology — the study of animal behaviour — and to look at the actual behaviour of mammals. What he found was a set of behavioural systems that were universal across the species and that had clear functions: **attachment**, which provided a safe base from which to explore and a refuge from threat; **feeding**, which was also the physiological system for the growth of the body; and **avoidance and exploration**, which constituted the other half of the human behavioural repertoire.

The functional argument was that the attachment system is **selected for** because of its consequences: infants attached to their mothers survived. The claim of the 1960s papers was that the attachment relationship was a **primary** system, a species-typical system present from birth, biologically prepared, and not a derivative of feeding.

## 1.2 The critical period and the monotropy hypothesis

Bowlby's developmental claims were the ones that caused the most controversy. He proposed a **critical period** in the first eighteen months to two years during which the attachment relationship is formed, and a **sensitive period** thereafter in which the relationship can still be formed but more rigidly. And he proposed **monotropy** — that the infant forms a single, primary, exclusive attachment, normally to the mother, and that other relationships are secondary.

The critical period claim was attacked on the grounds that it implied that separation after two years was catastrophic, and this was in turn attacked on the grounds of the effects of the orphanage studies of the 1940s and 1950s, which showed that children raised in institutions without a consistent caregiver developed profound difficulties. Bowlby's rejoinder was that the institution's failure was not the timing but the absence of a consistent figure, and the distinction matters.

## 1.3 The controversies

Three controversies dominated the field and all three are still live.

**The nature of the attachment categories.** Whether attachment is categorical — a small number of qualitatively distinct patterns, as in the strange situation — or dimensional — a continuum of scores, as in most of the self-report measures — remains unresolved, and the resolution is a matter of the measurement instrument rather than of the theory.

**The cross-cultural universality of the patterns.** The strange situation was developed in a specific social context, and its cross-cultural use produced a well-known problem: the pattern that looked "avoidant" in one setting was the pattern that looked "secure" in another, in which the caregiver's response to a stranger was appropriate. The resolution was the recognition that the strange situation measures the *child's* response to the reunion, and that the *caregiver's* response must be coded separately, and that the classification is of the

child, and that the categories do not have the same meaning in every setting. This is an important methodological lesson and it is directly relevant to the cultural analysis in Section 11.

**The relationship between the child and the adult measures.** The Adult Attachment Interview measures a quite different construct from the strange situation: it measures the discourse of an adult about their own attachment history, and the categories it produces (secure, preoccupied, dismissing, and the unresolved category) have a rather different meaning from the child's categories, and the correlation between the two is modest. Practitioners who treat the two as interchangeable are making an error.

### 1.4 The move to the clinic

Attachment research moved into clinical practice through several paths. The **attachment-based family therapy** developed by the Attachment and Biobehavioral Catch-up (ABC) group, and the **Child and Family Traumatic Stress Intervention (CFTSI)** of the same group, translated the research into parent-focused interventions for young children who have been maltreated. The **dyadic developmental psychotherapy (DDP)** of Linehan's group, described in Chapter 42 in relation to mentalization, translated the attachment findings into a treatment for young people with attachment disruption, including those with a borderline personality disorder. And the **Mentalization-Based Treatment** of Fonagy and Bateman, described in Chapter 42, was built on the attachment research and has become the most substantial evidence-based treatment for adolescent borderline personality disorder.

## 2. The Core Theory

### 2.1 Attachment as a behavioural system

**Attachment** is a behavioural system organised around the child, activated by the child's need for protection, and functioning to provide a secure base from which to explore and a refuge from threat. The system's functions are:

**Protection.** The attachment figure provides a refuge in the face of threat, and the child's proximity to the figure reduces the physiological arousal associated with threat.

**Proximity maintenance.** The system is activated by separation and by threat, and it produces the child's seeking of the figure.

**A secure base.** The system promotes exploration from the point of security, and the exploratory behaviour is more extensive and more effective when the secure base is available.

**An affect regulation system.** The child's capacity to manage negative affect is a function of the availability of the attachment figure, and the internalisation of that capacity over the first years is the developmental foundation of the regulation of affect throughout life.

**A basis for the self.** Bowlby and his colleagues proposed that the attachment relationship is the setting in which the sense of self is constructed, and the two are complementary: the child's model of the other as a reliable or an unreliable person and the child's model of the self as worthy or unworthy of care are developed together.

### 2.2 The conditions of attachment

Three conditions are necessary for the formation of an attachment, and all three are necessary and none is sufficient.

**Availability.** The caregiver must be available and responsive.

**Sensitivity.** The caregiver must be sensitive to the child's signals, and the sensitivity must be accurate rather than perfect, because a caregiver who responds to every signal in a synchronised way produces a child with no capacity to manage the states that arise when the caregiver is unavailable.

**Affective investment.** The caregiver must be affectively invested in the child, and the child must be able to recognise that the caregiver is invested in them.

The three conditions can be separated empirically, and the separation is clinically important. A caregiver can be available and insensitive; a caregiver can be sensitive and not invested; a caregiver can be invested and unavailable. Each produces a different pattern, and each requires a different intervention.

### 2.3 Attachment and the other behavioural systems

Attachment operates in relation to the other systems and is not the only influence on behaviour.

**The exploratory system** is inhibited by attachment activation, and the child who is constantly anxious will not explore, and the exploration deficit is a common presentation.

**The caregiving system** is activated by the young person's cues and is suppressed by the young person's anger and by the caregiver's own unresolved loss. A parent whose own attachment history is unresolved may be unable to provide the caregiving that the young person cues for, and the young person's angry protest is exactly the signal that suppresses the caregiving system.

**The sexual system** develops through puberty and is not a derivative of the attachment system, and its interaction with the attachment system in adolescence produces the specific difficulties of the sexuality conflict described in Chapter 33.

## 3. The Internal Working Model

### 3.1 The construct

The **internal working model** is the set of expectations, drawn from the primary relationship, about the availability and the trustworthiness of others, about the response of others to one's own needs, and about the worthiness of the self. It is a cognitive schema that operates largely outside awareness, and it is both an interpreter of new experience and a generator of behaviour.

The three components are as follows.

**A model of the attachment figure** — as available or unavailable, as responsive or unresponsive, as trustworthy or not.

**A model of the self** — as worthy of care or not, as able to elicit care or not, as having a right to need or not.

**A model of the interaction** — whether the seeking of comfort works, whether the protest produces response, and what happens when the figure is unavailable.

### 3.2 The developmental trajectory

The working model is built in the first two to three years, and it is then **revised** rather than replaced.

**From birth to three years**, the model is built from the primary relationship. It is a relatively simple, affective, and non-verbal schema, and it is formed by the repeated experience of the infant's signals being answered.

**From three to six years**, the model is elaborated. The child can now hold a mental representation of the caregiver, can anticipate responses, can begin to use it to plan behaviour, and can develop the fantasy and the reality as separate systems.

**From six to eleven years**, the model becomes increasingly deliberate and increasingly capable of being reflected upon, and the child can be argued with, and this is a direct consequence of the cognitive development of the period.

**From eleven onwards**, the model becomes a more organised and more differentiated system, capable of multiple relationships, of ambivalence, and of reflection, and the adolescent's task is the *revision* of the model rather than its formation, and the revision is what the developmental literature calls the *secondary revision* of the working model.

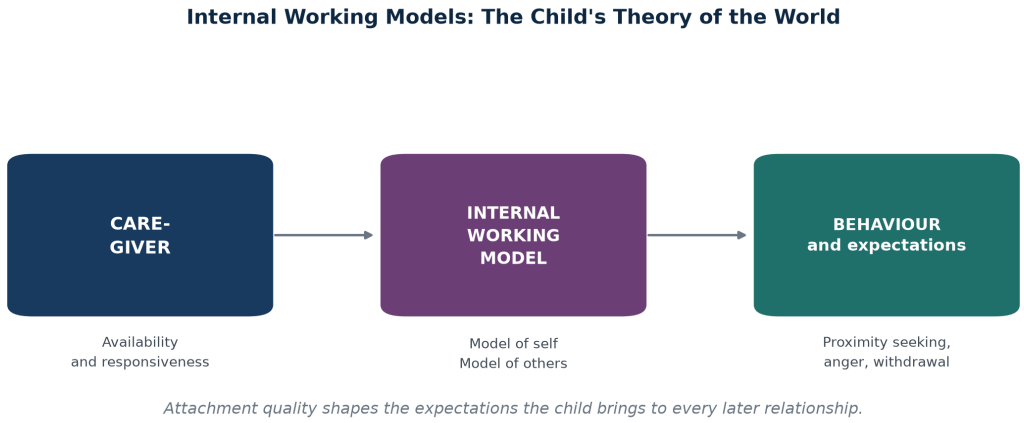



**Figure 34.23** The internal working model represented as a schema with three components — a model of the attachment figure, a model of the self, and a model of the interaction — and the developmental revisions that occur across the life span.

## 3.3 The revision in adolescence

Two processes operate in adolescence and both are clinically important.

**The revision of the working model.** The adolescent's developmental task includes moving from the primary relationship to a wider world of relationships, and this requires a revision of the working model that permits multiple relationships of different kinds. The adolescent who has a working model in which all relationships are of the same kind and carry the same expectations cannot make the move, and the young person who remains in a pattern of intense and exclusive relationships is often presenting this.

**The acquisition of the capacity to sustain a relationship through a period of developmental change.** The working model of the childhood is a model of a relationship in which the other is stable. The adolescent's relationships are with people who are also changing, and the working model must be revised to accommodate the mutability of the other.

The clinical relevance is that an adolescent with an insecure working model will be at their most vulnerable precisely at the developmental point at which the working model is under revision, and a practitioner who interprets the adolescent's relationship difficulties as a regression is making a serious error.

# 4. The Attachment Patterns

## 4.1 The child categories

The categories derive from the **strange situation**, a laboratory procedure in which the child's behaviour is observed across a series of eight episodes in which the caregiver and a stranger are present or absent.

**Secure.** The child explores readily, is distressed on separation, is readily comforted on reunion, and is not markedly avoidant of the caregiver.

**Avoidant (insecure-avoidant).** The child does not distinguish between the caregiver and the stranger, explores readily even during separation, shows little distress at separation, and avoids the caregiver on reunion. The avoidant pattern is a defensive strategy: the child's experience of the caregiver's availability is unpredictable or the caregiver's responses are rejecting, and the child has learned that the reliable response to needing is to not need.

**Ambivalent or resistant (insecure-ambivalent).** The child is preoccupied with the caregiver, explores little, is highly distressed at separation, is angry on reunion, and is difficult to comfort. The ambivalent pattern reflects a history of caregiver behaviour that is unpredictable and inconsistently responsive, so that the child is angry about the separation and is also unable to use the caregiver on reunion.

**Disorganized.** The child shows contradictory, frozen, or dazed behaviour in the caregiver's presence, and the behaviour of the caregiver is the key to understanding it. The disorganized pattern is the one associated with maltreatment and with the loss of a caregiver, and it is described in the clinical literature as a **fright without solution**: the child approaches a caregiver who frightens the child, and the child has no organised strategy for this.

## 4.2 The adult categories

The adult categories derive from the **Adult Attachment Interview**, which is a semi-structured interview about the adult's own childhood and its significance, and which is scored for the quality of the discourse rather than for its content. The categories are:

**Secure (autonomous).** The narrative is coherent, the account of the difficulties is not accompanied by a defensive dismissal, and the person can describe the effects of their history without being overwhelmed.

**Preoccupied (anxious).** The narrative is detailed and often vivid, and the account of the past is marked by unresolved conflict, by a sense of the parent's own needs intruding, and by a difficulty in moving on from the past.

**Dismissing.** The account is minimised, the difficulties are attributed to the child or to the circumstances, and the person presents as self-sufficient and uninterested in the discussion.

**Unresolved (or preoccupied/unresolved).** The discourse shows lapses in monitoring, disorganised or contradictory accounts, and unusual responses to the losses in the person's history, and this category is associated with unresolved grief and with trauma.

The critical point for the practitioner is that these categories are *not* the adult versions of the child's categories, that they are derived from a different instrument measuring a different construct, and that the two are only modestly correlated.

### 4.3 The attachment matrix

A useful clinical instrument combines the two: the **attachment matrix** plots the young person's behaviour in the session against the attachment signals the practitioner observes in the caregiver, and the two are read against each other.

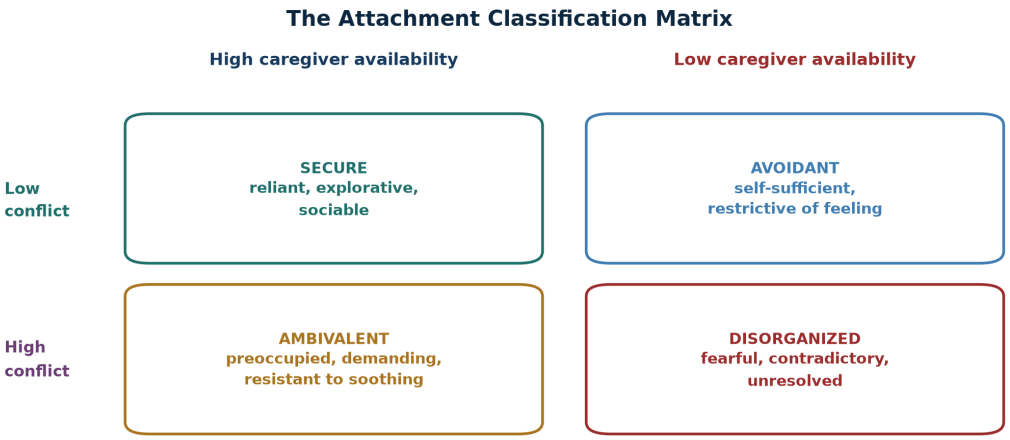



**Figure 34.24** The attachment matrix, plotting the young person's behaviour in the session against the attachment signals shown by the caregiver, with the four quadrants labelled.

The reading is as follows. A young person who is **avoidant in the session** with a caregiver who is **preoccupied** is a young person who has learned to manage the caregiver's anxiety, and the intervention is with the caregiver. A young person who is **preoccupied** with an **avoidant** caregiver is a young person whose bid is being met by withdrawal, and the intervention is to introduce a consistent and responsive source of care, which is frequently a professional. A young person who is **disorganised** with a caregiver who is **unresolved** is the classic maltreatment presentation, and the intervention is the most difficult in the field.

## 5. Attachment-Based Therapy

### 5.1 The principles

The attachment-based therapies share five principles, and a student should be able to state them.

**The relationship is the treatment.** The therapy is directed at the development of a relationship in which the young person can experience the caregiver as available and responsive, and the therapeutic relationship itself is the vehicle through which the working model is revised.

**The caregiver is the target of the intervention.** The work is with the caregiver's responsiveness, because the young person's working model is built from the primary relationship, and a change in the caregiver's availability is a change in the material from which the model is built.

**The developmental task is respected.** The work does not treat the young person's behaviour as a problem to be extinguished but as an attempt to manage a situation that has been overwhelming, and the developmental appropriateness of the intervention is the requirement.

**The intervention is the creation of conditions in which the attachment system can be used.** In practice this means the removal of the conditions that suppress the attachment system — the anger that suppresses the caregiving system, the inconsistent response that teaches the child to manage the caregiver's anxiety, the maternal behaviour that teaches the child to self-soothe.

**The treatment of the caregiver's own attachment history is a component.** A parent with an unresolved attachment history will reproduce the pattern, and the work with the parent's history is part of the work with the child.

## 5.2 The variants

**Attachment and Biobehavioral Catch-up (ABC)** is a parent-mediated intervention for infants and toddlers who have experienced maltreatment or disrupted caregiving. The core procedure is the video-feedback intervention: the parent is filmed interacting with the child and then watches the recording with the practitioner, who helps the parent to see the child's bids and their own responses. The evidence base is good, and the mechanism is understood.

**Child and Family Traumatic Stress Intervention (CFTSI)** extends the ABC work to the treatment of trauma in children aged three to six and to their caregivers, combining the video-feedback with the teaching of specific skills: the recognition of the child's trauma-related behaviour, the reduction of the child's avoidance, the parent's own regulation, and the development of the child's attachment behaviours.

**Dyadic developmental psychotherapy (DDP)** is a parent-child attachment intervention for young people with attachment trauma and for those with emerging or established borderline personality disorder. Its distinguishing feature is the **PACE** stance — Playfulness, Acceptance, Curiosity, and Empathy — and its technique is the "follow your lead" responsive interaction in which the practitioner enters the child's world and reflects back the child's own intentions with delight. The evidence base is good and it is the most substantial of the attachment-based treatments for the adolescent population.

**Mentalization-Based Treatment** is described in Chapter 42 and is the largest evidence base of all the attachment-based approaches for adolescent borderline personality disorder.

**The general principles applied without a protocol** — the practitioner working with the caregiver's responsiveness, with the recognition of the young person's bids, with the repair of ruptures, and with the parent's own history — are the practice that most practitioners in this book will actually need, and they are what the rest of this chapter describes.

## 5.3 The practices

**The recognition of the bid.** A bid is a small, usually nonverbal, signal that the young person is seeking proximity, contact, or reassurance. The practitioner's task in the first sessions is to learn to see them, and the technique is the same as the technique used in the ABC video-feedback: the practitioner observes, names

what the young person did, and names what it meant.

**The response.** The response must be prompt, and it must be congruent, and it must be about the bid and not about the behaviour. A young person who is angry at being told what to do is not making a bid and should not be responded to as one.

**The repair.** The most important single practice. When the practitioner misses a bid — and the practitioner will, and the young person will know — the repair is the opportunity. The practitioner names what happened, acknowledges the young person's response, and does not require the young person to reassure them. A practitioner who can repair a rupture with a sixteen-year-old has demonstrated more than a practitioner who has never missed one.

**The prohibition on the use of the therapeutic relationship to produce compliance.** This is a critical ethical point in the attachment-based work and it is stated explicitly in the CFTSI and the DDP protocols: the practitioner does not use the attachment relationship to secure the young person's cooperation, and the practitioner does not withhold to secure it. The attachment figure must be available precisely when it is not needed for the practitioner's purposes.

**The consistency of the response.** The response must be the same every time, and the "every time" includes the times when the practitioner is tired, and this is why the work is best done in a team.

**The work with the caregiver's history.** The parent's unresolved grief, the parent's own experience of being cared for or not, and the parent's own unresolved losses are worked with, usually in a separate strand, and the practitioner should expect that a parent whose own history is unresolved will struggle to respond consistently.

## 6. Further Reading for This Chapter

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# 7. Attachment-Based Work with Children and Adolescents

## 7.1 The developmental fit

Attachment theory is unusually well matched to the adolescent period, and the reasons are worth stating precisely.

**Adolescence is the period of the working model's revision.** The adolescent's developmental task includes the revision of the expectations about others that were formed in the first three years, and the revision is possible only in the context of a relationship that is safe enough to revise in.

**Adolescence is the period of the secondary attachment systems.** The peer relationships, the teacher relationships, and the relationships with non-parental adults become the significant developmental relationships, and the adolescent's working model is revised in the context of these.

**The attachment system is at its most activatable in adolescence.** The hormonal and social changes of the period increase the salience of attachment cues, and the adolescent who is in an insecure state is at the point at which the system is most easily reorganised and also most easily overwhelmed.

**The developmental task of separating from the parents is an attachment task.** The detachment from parental control described in Chapter 33 is the developmental renegotiation of the attachment relationship, and it is easier where the working model is secure, because a secure working model permits the adolescent to be autonomous without losing the relationship.

## 7.2 The presentations

Four presentations are common in adolescent attachment work, and the practitioner should be able to recognise each.

**The preoccupied adolescent.** The young person is intensely involved with the parents and with the practitioner, is anxious at separation, is angry on reunion, and is unable to tolerate the distance that the work requires. The underlying model is that the other is available but unreliable and that needing produces anxiety, and the intervention is the consistency of the response, and the introduction of a source of care that does not withdraw.

**The dismissing adolescent.** The young person is self-sufficient, deflects, does not seek help, describes the parents as fine and their own difficulties as nobody's business, and is subtly contemptuous of emotional display. The underlying model is that needing is dangerous and that the self is worthless as a source of comfort, and the intervention is the repeated, patient demonstration that the need will be met, which takes a long time and requires the practitioner to be willing to be unhelpful in order to be reliable.

**The disorganised adolescent.** The young person's behaviour in the session is contradictory: she approaches and then withdraws, or is simultaneously warm and hostile, or is dissociate and present at the same time. The underlying pattern is the absence of an organised strategy, and the clinical significance is that the young person's behaviour cannot be interpreted as communication, and the practitioner must not try.

**The young person with no stable attachment figure.** The young person has had a series of caregivers and has no figure to whom the attachment system is directed, and the system is consequently unattached and is available for attachment to a new figure. This is the young person for whom the therapeutic relationship may be the first secure base, and it is also the young person for whom the abandonment of the therapeutic relationship will be most damaging, and the termination planning of Chapter 58 is correspondingly important.

## 7.3 Working with the caregiver

The work with the caregiver has five components, and they are the substance of the attachment-based practice.

**The observation of the interaction.** The practitioner watches the young person and the caregiver together, and the practitioner's task is to see the bids and the responses. This is the technique of the ABC video-feedback and it is the technique of the observation in the DDP model, and it is learnable.

**The naming of the bids.** The practitioner names, in the session, what the young person did and what it meant: "she moved her chair closer to you three times in the last ten minutes, and each time she did it she also looked at the door." The naming is done with the caregiver, in a way that is observational rather than evaluative, and the shift from "she is attention-seeking" to "she is seeking you and checking whether you are available" is the single most useful clinical move in the work.

**The consistency of the response.** The caregiver is helped to respond promptly and congruently, and the specific guidance is given about the two common failures: the response that is delayed and the response that is inconsistent.

**The repair.** The caregiver is taught to repair ruptures with the young person, and the repair is demonstrated and rehearsed, because most caregivers have never repaired a rupture with their own parent and do not know that it is possible.

**The caregiver's own history.** Where the caregiver has an unresolved history, the work on it is a component, and it may need to be referred to a clinician who can work with an adult's attachment history, because it is a different set of skills from those of the adolescent work.

### 7.4 The young person's developmental work

**The recognition of the need.** Much of the work with the insecure adolescent consists of helping the young person to notice their own bids, which is a considerable achievement for a young person who has learned that bids are met with rejection or with withdrawal.

**The tolerance of the need.** The young person must also learn to tolerate the need — to sit with the discomfort of needing, without immediately doing something to end it. This is the developmental work of the secure base.

**The testing.** The young person will test the practitioner, and the tests will look like provocation or sabotage. The tests are of the same kind as the tests of the infant, and the practitioner's task is to respond in a way that is congruent and non-defensive, and to notice afterwards what the test was for.

**The multiple relationships.** The developmental task of moving from the primary relationship to a wider world of relationships is supported concretely: the young person is helped to find and to sustain other relationships, in the peer world, with other adults, and the practitioner supports rather than blocks this.

### 7.5 The relationship with the practitioner

The ethical and clinical requirements of the attachment relationship with the practitioner are strict, and a student should be able to state them.

**The practitioner is available and consistent.** This is the basic requirement, and it is the hardest to provide in a service with short contacts and high turnover, and the services with the highest turnover are the ones with the most attachment-disrupted young people.

**The practitioner does not withhold.** The practitioner does not use the relationship to secure compliance, and does not withdraw to produce a change, and does not withhold to teach a lesson.

**The practitioner does not require reassurance.** The young person is not required to reassure the practitioner, and the practitioner does not ask for it, and the asking is a common error in inexperienced practitioners.

**The practitioner repairs.** The practitioner names the rupture and does not require the young person to repair it.

**The practitioner is honest about the limits.** The practitioner tells the young person about the end of the therapy, about the limits of their availability, and about what will happen if the young person is in crisis. The honesty is itself an attachment experience, and it is different from the experience of a practitioner who is reliable in the sessions and absent in the crisis.

## 8. Case Illustration: The Young Woman Who Had Learnt to Be Left

**Presentation.** Nadia is sixteen and has been placed in foster care for the second time in two years. She is referred by the school with "attachment difficulties, refuses to engage, and has already said she will not be here long." The placement is with a family she has met twice.

**The first session.** Nadia is late, and she is polite, and she is careful, and the practitioner notices the following things and writes them down. Nadia asks, in the first five minutes, how long the sessions are. She asks whether the practitioner will be there next week. She says, in the middle of the session, "I don't really

do the talking thing, you should know that now." And she does not, at any point in fifty minutes, refer to anything that has happened to her.

The practitioner also observes Nadia's body, because the practitioner was trained to observe bodies, and Nadia's shoulders are raised and her voice is pitched slightly high and her hands are busy with the sleeve of her jumper throughout the session.

**The attachment matrix.** In the first family session, the matrix is read. Nadia is **dismissing** in the session: she does not seek the foster parents, does not look at them, answers their questions briefly, and presents as a young woman with no needs. The foster mother is **preoccupied**: she talks a great deal, she explains the house rules, she asks Nadia repeatedly how she is settling, she says "we just want you to feel at home", and she is visibly hurt by Nadia's flatness. And the practitioner observes the pattern in the room: the foster mother asks a question, Nadia gives a minimal answer, the foster mother asks again, and the second question is more emotional, and Nadia becomes more closed.

The matrix reading is: **the young person is managing the adults' anxiety, and she is doing it by being self-sufficient.** The foster mother's anxiety produces the second question, the second question produces the withdrawal, and the withdrawal produces the foster mother's hurt, and the hurt produces more checking. Nothing in the system is a behaviour problem. Everything in the system is the attachment system in operation, in a young person who learned in her first fourteen years that the reliable response to an adult who is anxious is to stop needing anything.

**The formulation.** The working model, as the practitioner reconstructs it from the school records, the previous placement's notes, and Nadia's own fragmentary account, contains a model of the other as unavailable and a model of the self as not worth the trouble. The specific history is not yet known and the practitioner does not invent it; the first task is to establish it, carefully, over time, and at Nadia's pace.

**The intervention.** Five elements.

*Element one: the foster parents' preparation.* Before the second session, the practitioner meets the foster parents alone and explains the attachment matrix reading in plain terms: "Nadia is not being cold. She is being careful. And the reason she seems cold is that she has learned that when adults worry, they ask again, and when they ask again, they get upset, and when they get upset she ends up having to manage their feelings." The practitioner tells them what the work will require: consistency, patience, a small number of bids being answered, and a great deal of repetition before anything changes. The foster mother cries at this point, and the practitioner does not interpret the crying, and this is itself a demonstration of the practice.

*Element two: the change in the foster parents' responding.* The specific instruction is: "Ask once. If she answers, that is the end of the question. If she says nothing, that is also the end of the question. And then, in the evening, tell her one thing you noticed about her that is not about school or about the house rules — that she ate all of the lasagne, that she was reading on the sofa, that she laughed at something — and then stop talking." The purpose of the evening message is to establish that the contact continues when the young person has not initiated it, without the young person having to do anything to end it.

*Element three: the placement question.* The practitioner addresses the placement's framing directly. The statement in the school referral that Nadia "will not be here long" is a prediction, and the prediction is a self-fulfilling one, and the practitioner says so to the team: "If we expect her to leave, we will read everything she does as evidence that she is leaving, and she will pick that up in about a week." The team agrees to a review at three months rather than at six weeks.

*Element four: the individual work.* The work with Nadia is slow and consists of three things. First, the practitioner is boring and reliable: the same time, the same room, the same ten minutes of talking about nothing at the start. Second, the practitioner names the bids when they occur, gently and without pressure: "You asked me twice whether I would be here next week." Third, the practitioner does not require any disclosure. The practitioner explicitly says: "You do not have to tell me anything about what has happened. That is not what this is for. What I am for is being here."

*Element five: the developmental work.* Two pieces. The first is the finding of an adult outside the family: a youth worker at the school, a specific and reliable person, and the practitioner introduces them and then steps back. The second is the introduction of the peer world, which Nadia has given up entirely, and the work is to get one, low-demand social activity — a youth group, a sports session, a shift at the school café — with the specific and deliberate aim of giving her a relationship with a peer that is not a performance.

**The outcome.** At three months Nadia has not disclosed anything about her history, and the practitioner regards this as a good outcome and says so to the team. She has stopped asking whether the practitioner will be there. She has come to three of the youth group sessions and has spoken to two of the other young people. The foster mother reports that "she's still not affectionate but she comes down for dinner now, and she told me about a song."

**The interpretation.** The case illustrates the attachment framework in practice and it makes two points that a student should take away. The first is that the young person's "refusal to engage" was a competent and adaptive response to a history in which engaging was dangerous, and the label was describing the coping and calling it the pathology. The second is that the most important clinical event in the three months was not a disclosure and was not a breakthrough; it was the provision of a predictable adult and the removal of the double question, and the durable change in a young person with a disrupted attachment history is built out of exactly that.

# 9. Evidence and Research Support

## 9.1 The status of the evidence

Attachment theory has a large and rigorous research base, and the evidence for the interventions derived from it is good but narrower than the theory.

**The attachment research is highly reliable.** The strange situation and the Adult Attachment Interview are among the best-validated measures in the developmental sciences, with high inter-rater reliability, extensive normative data, and a large body of longitudinal research. The prediction of attachment classification into adulthood is among the more robust findings in the field.

**The longitudinal findings are strong.** Attachment classification in infancy predicts a range of outcomes at adolescence and in adulthood, with the largest effects in the socially and emotionally at-risk groups, and the effects are largest for the insecure classifications.

**The ABC intervention has good support** for infants and toddlers, and the video-feedback mechanism is understood.

**The CFTSI has good support** for young children with trauma and for their caregivers.

**The DDP has good support**, with a randomised controlled trial showing significant reductions in self-harm in the borderline population, and a body of mechanistic work on the PACE stance and on the development

of the mentalizing capacity.

**Mentalization-based treatment has the strongest evidence base** of all the attachment-based approaches for adolescent borderline personality disorder, and Chapter 42 develops it.

**The evidence for attachment-based work with the broader adolescent population is thin.** This is the honest position: a young person with an insecure attachment pattern and no diagnosed disorder is unlikely to be referred to an attachment-based treatment, because the presentation is not severe enough, and the evidence for the general application of attachment principles to adolescent presentations is limited to the clinical trials of the specific protocols.

### 9.2 Limitations of the evidence

**The measures are Western and the categories may not transfer.** The cross-cultural literature is substantial and the most serious methodological finding is the context-dependence of the caregiver's behaviour in the strange situation, and the field's resolution of it is partial.

**The adult and the child measures are not the same construct**, and the moderate correlation between the two has been over-interpreted in the clinical literature.

**Attachment classification is a group-level description**, and the predictive power at the level of the individual is considerably weaker than the group-level figures suggest, and this is a point of statistical sophistication that is important for a practitioner who is being told that a young person's category predicts their future.

**The intervention research is concentrated in infancy and early childhood** and in the borderline population, and the adolescent range is comparatively under-researched.

**The outcomes measured are largely behavioural and attachment-related**, and the mental health and the quality of life outcomes of the attachment-based interventions are less extensively reported.

## 10. Strengths and Limitations of the Attachment Perspective

### 10.1 Strengths

**It is developmentally founded and empirically supported.** The theory is not a metaphor and its measures are among the best in the field.

**It explains why the same young person behaves differently with different adults**, which is one of the most practically useful things a theory can do for a practitioner.

**It provides a formulation of the young person as adaptive**, which is both more accurate and more useful than a formulation of the young person as defective.

**It is practical.** The bids, the responses, the consistency, and the repair are learnable and they are usable in any modality.

**It locates the target in the relationship**, which is where the developmental evidence points.

### 10.2 Limitations

**It under-accounts for the individual.** A young person with a neurodevelopmental condition, with a psychotic illness, or with a specific learning difficulty has difficulties that the attachment account does not explain.

**The categories are Western and their universality is contested**, and the practitioner who applies them cross-culturally without adjustment will misdescribe.

**The adult categories are frequently misapplied to young people**, and the adult attachment interview categories do not translate into the child categories.

**The relationship between attachment and the individual-differences measures is weaker than the clinical literature implies**, and the attachment story has sometimes been used to explain more than it can.

**The interventions are demanding.** The DDP requires a trained team and a stable placement, and the CFTSI requires trained parents and a video-feedback setup, and the services in which the young people who most need this work are found are frequently not resourced for it.

# 11. Cultural and Ethical Considerations

## 11.1 The cultural specificity of the attachment categories

The cross-cultural literature on attachment is one of the most methodologically instructive in the developmental sciences, and a student should know its central finding.

**The strange situation is not culturally neutral.** It was developed in a specific social context, and the classification depends on the cultural meaning of the stranger's behaviour and on the caregiver's familiarity with the procedure.

**The caregiver's response must be coded separately.** The most important methodological correction was to recognise that the strange situation measures the child's response to the reunion and that the caregiver's response must be coded independently, and that a caregiver's response that is coded as "insecure" in one setting may be coded as "secure" in another, because the meaning of the behaviour differs.

**The distributions differ across cultures.** Some studies have found higher proportions of the insecure classifications in certain settings, and the interpretation of these differences is contested, and the field's position is that the differences may reflect the circumstances of caregiving in those settings — the instability of the caregiving arrangements, the pressure on the caregiver, the absence of supports — rather than a deficiency in the caregivers.

**The categories may be a poor fit where caregiving is collective.** Where the primary caregiving figure is a kin network rather than an individual, the strange situation's assumptions about the singular attachment figure and about the exclusive relationship do not hold, and a practitioner who applies the categories will misdescribe the family.

**Practices that are optimal for attachment are not universally optimal.** The practices that produce a secure attachment — the consistent and sensitive response, the reliable caregiver, the stable routine — are good things in most circumstances, and in a setting where the family is under severe pressure they are additional demands on a caregiver who is already overwhelmed, and the clinical judgement required is to support the caregiver's capacity before expecting the practice of it.

## 11.2 Ethical considerations

**The therapeutic relationship as an attachment relationship.** The ethical status of the attachment relationship with the practitioner is a live question. The attachment-based approach treats the relationship as the treatment, which is powerful and which raises a question about the nature of the obligation when the relationship ends. A practitioner who has been the young person's secure base has a duty of care in the ending that goes beyond the ordinary termination, and the practice of the attachment-based approaches is to prepare for the ending from the beginning.

**The prohibition on the use of the relationship to secure compliance.** This is stated explicitly in the protocols and it is a genuinely different ethical position from the one in the behavioural tradition, and the two traditions can be in direct conflict in a case. A practitioner working with the behavioural approach of Chapter 26 and the attachment approach of this chapter must resolve the conflict explicitly, and the resolution will usually be that the attachment relationship is not to be used to produce the behaviour change, and that the behaviour change is not to be produced at the cost of the relationship.

**The status of the caregiver in placements.** A young person in a placement has a legal guardian who is not the attachment figure, and the conflict between the guardian's authority and the attachment relationship is a real and frequently mishandled difficulty.

**The young person's right to refuse.** The attachment approach must not become a means of compelling disclosure. A young person who declines to talk about her history has the right to do so, and the practitioner who pursues disclosure as a condition of the work has turned the secure base into a condition.

**Cultural and religious considerations in the caregiver's response.** The practices that the attachment literature recommends may conflict with a family's religious or cultural practices, and the practitioner should establish what those practices are before the intervention, and should not require the family to abandon them.

**Confidentiality and the caregiver's access.** The attachment-based approach requires the practitioner to share observations with the caregiver, and this must be negotiated explicitly with the young person.

**The avoidance of re-traumatisation.** The work with a young person with a disrupted history must not require the young person to revisit the history in order to prove that they have worked, and the practitioner must be prepared for the young person's own decision about how much to disclose, including the decision to disclose nothing.

## 12. Key Terms

### Key Terms at a Glance

- **Attachment** — A behavioural system organised around the child, activated by the need for protection, and functioning to provide a secure base for exploration and a refuge from threat.
- **The functions of the attachment system** — Protection, proximity maintenance, a secure base for exploration, affect regulation, and the basis for the self.
- **The behavioural control systems** — Attachment, feeding, and avoidance and exploration, each with its own functions and its own interaction with the others.
- **The three conditions of attachment** — Availability, sensitivity, and affective investment; all necessary and none sufficient.
- **Sensitivity** — The caregiver's accurate responsiveness to the child's signals, as distinct from a perfectly synchronised responsiveness, which produces a child with no capacity to manage the states that arise when the caregiver is unavailable.
- **Affective investment** — The caregiver's emotional investment in the child, and the child's recognition of it.
- **The critical period** — The period in the first eighteen months to two years during which the attachment relationship is formed.
- **The sensitive period** — The period after the critical period in which attachment can still be formed, but more rigidly.
- **Monotropy** — Bowlby's proposal that the infant forms a single, primary attachment and that other relationships are secondary.
- **The secure base** — The function of the attachment figure as the platform from which the child explores and to which the child returns.
- **Internal working model** — The set of expectations drawn from the primary relationship about the availability of others, about the response to one's needs, and about the worthiness of the self.
- **The three components of the working model** — A model of the attachment figure, a model of the self, and a model of the interaction.
- **The developmental trajectory of the working model** — Formation in the first three years, elaboration to six, deliberate reflection from six to eleven, and organised differentiation from eleven onwards.
- **The secondary revision of the working model** — The adolescent's revision of the working model, permitting multiple relationships of different kinds and mutability in the other.
- **Strange situation** — The laboratory procedure developed by Ainsworth and colleagues, consisting of eight episodes in which the caregiver and a stranger are present or absent.

### Key Terms at a Glance (continued)

- **Secure attachment** — The child who explores readily, is distressed on separation, is readily comforted on reunion, and does not avoid the caregiver.
- **Insecure-avoidant attachment** — The child who does not distinguish the caregiver from the stranger, shows little separation distress, and avoids the caregiver on reunion; a defensive strategy against unreliable availability.
- **Insecure-ambivalent attachment** — The child who is preoccupied with the caregiver, explores little, is highly distressed at separation, and is angry on reunion.
- **Disorganized attachment** — The contradictory, frozen, or dazed behaviour in the presence of a caregiver whose behaviour is frightening or whose history is unresolved; the "fright without solution".
- **Adult Attachment Interview** — The semi-structured interview about the adult's own childhood, scored for the quality of the discourse and producing the adult attachment categories.
- **Secure, preoccupied, dismissing, and unresolved** — The four adult attachment categories, derived from a different instrument from the child categories and only modestly correlated with them.
- **The attachment matrix** — A clinical instrument plotting the young person's behaviour in the session against the caregiver's attachment signals, read in four quadrants.
- **Attachment-based family therapy (ABC)** — The parent-mediated intervention for infants and toddlers, using video-feedback to teach the caregiver to see the child's bids.
- **Child and Family Traumatic Stress Intervention (CFTSI)** — The extension of the ABC work to the treatment of trauma in young children aged three to six and to their caregivers.
- **Dyadic developmental psychotherapy (DDP)** — The parent-child attachment intervention for attachment trauma and for emerging or established borderline personality disorder.
- **PACE** — The stance of playfulness, acceptance, curiosity, and empathy from which the dyadic developmental psychotherapy is delivered.
- **Follow your lead** — The responsive interaction technique in which the practitioner enters the young person's world and reflects back their intentions with delight.
- **The recognition of the bid** — The practice of noticing and naming the small signals by which a young person seeks proximity, contact, or reassurance.
- **Repair** — The practice of naming a rupture in the relationship and taking responsibility for it, without requiring the young person to reassure the practitioner.

**Key Terms at a Glance (continued)**

- **The prohibition on withholding** — The requirement that the attachment figure be available precisely when their availability is not needed for the practitioner's purposes.
- **The preoccupation with the small** — The developmental tendency of the adolescent period that makes the attachment system both more activatable and more reorganisable.
- **The four adolescent attachment presentations** — The preoccupied, the dismissing, the disorganised, and the young person with no stable attachment figure.
- **Attachment and the school context** — The application of the attachment principles to the school relationship, the classroom, and the school-based support.

## 13. Chapter Summary

Attachment theory is the developmental foundation of much of this book's practice, and its central claim is that attachment is a behavioural system with four functions: protection, proximity maintenance, a secure base, and affect regulation, and with the basis for the self as its developmental product. The chapter traced the history from Bowlby's break with psychoanalysis through the ethological argument, the critical period and monotropy proposals, and the three controversies that remain live — the categorical versus dimensional question, the cross-cultural universality of the categories, and the relation between the child and the adult measures.

The three conditions of attachment were described and their separability was noted, because the three failures produce three different presentations and require three different interventions. The internal working model was described in its three components and its developmental trajectory, with the adolescent's revision of the model identified as the developmental task of the period and the clinical error of interpreting the adolescent's relationship difficulties as a regression.

The attachment patterns were described in both the child and the adult categories, with the warning that the two derive from different instruments measuring different constructs and are only modestly correlated. The attachment matrix was described as a practical clinical instrument, with the four quadrants read and their interventions given.

The five principles of attachment-based therapy were stated, the five main variants were described with their evidence, and the five core practices were set out: the recognition of the bid, the response, the repair, the prohibition on the use of the therapeutic relationship to produce compliance, the consistency of the response, and the work with the caregiver's own history.

The work with young people was developed through the developmental fit, the four presentations, the five components of the work with the caregiver, the developmental work with the young person, and the strict requirements of the relationship with the practitioner, including the prohibition on requiring reassurance and the honesty about the limits of the availability.

The case of Nadia demonstrated the whole framework. The referral's "refusal to engage" was read, on the attachment matrix, as a young person managing the adults' anxiety by being self-sufficient, and the double question in the foster home was identified as the mechanism. The intervention was the preparation of the

foster parents, the change in their responding, the challenge to the placement's self-fulfilling prediction, the slow individual work that did not require disclosure, and the developmental work of finding an adult outside the family and a peer world. The outcome at three months was no disclosure and the practitioner treating that as a good outcome.

The evidence was presented with its structure: the attachment research is highly reliable and longitudinal, the intervention evidence is good but concentrated in infancy and in the borderline population, and the evidence for the general application of attachment principles to the adolescent range is thin. The limitations were named, including the statistical point that the prediction at the individual level is considerably weaker than the group-level figures suggest.

The cultural section addressed the cultural specificity of the categories, the context-dependence of the strange situation, the different distributions across cultures, the poor fit where caregiving is collective, and the point that the practices that are optimal for attachment are not universally optimal and may be additional demands on a caregiver who is already overwhelmed. The ethical section addressed the ethical status of the attachment relationship and the duty of care that persists beyond the ending, the prohibition on using the relationship to secure compliance and the resulting conflict with the behavioural tradition, the status of the guardian in a placement, the young person's right to refuse disclosure, the cultural and religious considerations in the caregiver's response, and the avoidance of re-traumatisation.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain Bowlby's methodological objection to psychoanalysis and the ethological response.
2. State the four functions of the attachment system.
3. State the three conditions of attachment and explain why their separability matters clinically.
4. Explain the relationship between the attachment, the exploratory, and the caregiving systems.
5. Define the internal working model and state its three components.
6. Describe the developmental trajectory of the working model.
7. Explain the secondary revision of the working model and the clinical error of reading the adolescent's relationship difficulties as a regression.
8. Describe the strange situation and state the three child attachment categories with their behavioural signatures.
9. Explain why the disorganized category is described as a fright without solution.
10. State the four adult attachment categories and explain why they are not the adult versions of the child categories.
11. Describe the attachment matrix and read its four quadrants.
12. State the five principles of attachment-based therapy.
13. Describe the ABC, CFTSI, DDP, and MBT approaches and their evidence bases.
14. State the five core practices of attachment-based work.
15. Explain the significance of repair and of the prohibition on requiring reassurance.
16. State the four reasons the attachment perspective fits the adolescent period.
17. Describe the four adolescent attachment presentations and their interventions.
18. State the five components of the work with the caregiver.
19. State the four requirements of the relationship with the practitioner.
20. Describe the case of Nadia and state the two lessons it teaches.
21. Summarise the status of the evidence for the attachment research and for the interventions.
22. State the five limitations of the evidence base.
23. State the five strengths and the five limitations of the attachment perspective.
24. Explain the central cross-cultural finding about the strange situation and the methodological correction it produced.
25. State the seven ethical considerations for attachment-based work.

## 15. Practice Activities

**Practice and Application**

### Activity 34.1 — The Attachment Matrix (75 minutes, small group)

In groups of three, observe — or reconstruct from a placement — a family session involving a young person and their caregiver. Code the young person's behaviour in the session on the secure, preoccupied, dismissing, and disorganised axes, and code the caregiver's on the secure, preoccupied, dismissing, and unresolved axes. Read the resulting quadrant and write the intervention that follows from it. Then write the specific observation that would make you revise the reading. The exercise teaches both the instrument and the humility that the instrument requires.

### Activity 34.2 — The Bid Diary (five days, individual)

For five days, record every small bid you can observe in one relationship — a family member's, a friend's, a colleague's — and what the response was. Record the bid, the response, and whether the response was prompt and congruent. Then, with a second person, swap diaries and read theirs, and identify the bids that you would have missed. The exercise builds the observation skill that the whole attachment-based practice depends on, and the second stage, reading another person's record, is where the learning of what a bid looks like actually happens.

### Activity 34.3 — The Repair Rehearsal (60 minutes, pairs)

One student plays a sixteen-year-old who has just been told something painful by the practitioner, and the other plays the practitioner who has mis-handled it. The young person withdraws. The practitioner's task is to repair: to name what happened, to take responsibility, and not to require the young person to reassure them. Then reverse roles. Then write down the three things the practitioner said, and assess each against the requirement that the repair not be a request for reassurance. Most practitioners, including the students, make the repair into a request for reassurance in the first attempt, and the exercise is designed to make that visible.

### Activity 34.4 — The Cultural Audit of a Category (60 minutes, individual)

Take a case from placement and apply the attachment categories, and then audit each judgement against the cultural and material context of the family. For every classification, ask: what does this caregiver's behaviour mean in this family, in this community, in this household, given the pressures they are under? Then write the revised formulation, using the caregiver's own account of the situation. The most common finding is that at least one of the four categories was an artefact of the practitioner's expectation rather than a description of the child, and the exercise is the most direct preparation for the cultural work in Chapter 48.

## 16. Annotated Reading Pathways

**For the theory.** Read Bowlby (1969) for the foundational statement and Bowlby (1988) for the clinical papers, which is the most useful single book for a practitioner. Then read Ainsworth and colleagues (1978) for the strange situation.

**For the developmental argument.** Read Cassidy and Shaver's handbook for the comprehensive state of the field, and Crittenden (2009) for the most usable developmental summary.

**For the clinical work.** Read the DDP manual for the PACE stance and the "follow your lead" technique, and read the CFTSI manual. Then read the video-feedback literature, which is the technique most applicable to practitioners who do not work with infants.

**For the evidence and the critique.** Read the longitudinal attachment studies, the DDP and MBT trials, and the critical reviews of the attachment measures. The critical reviews are essential: the field's own handling

of its instruments' limitations is a model of professional self-criticism.

**For the cultural question.** Read the cross-cultural attachment literature, especially the work on the strange situation's context dependence and on caregiving in non-Western settings, and read the literature on attachment in families experiencing poverty and migration. The cross-cultural material is not a supplement to the theory; it is part of the theory.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Attachment Theory and Attachment-Based Therapy** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is bowlby, Ainsworth, and the translation of attachment research into treatment. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the

next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tobias is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tobias's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tobias explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Tobias chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tobias has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions,

Tobias can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?

- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Attachment Theory and Attachment-Based Therapy

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Attachment Theory and Attachment-Based Therapy** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Attachment Theory and Attachment-Based Therapy** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

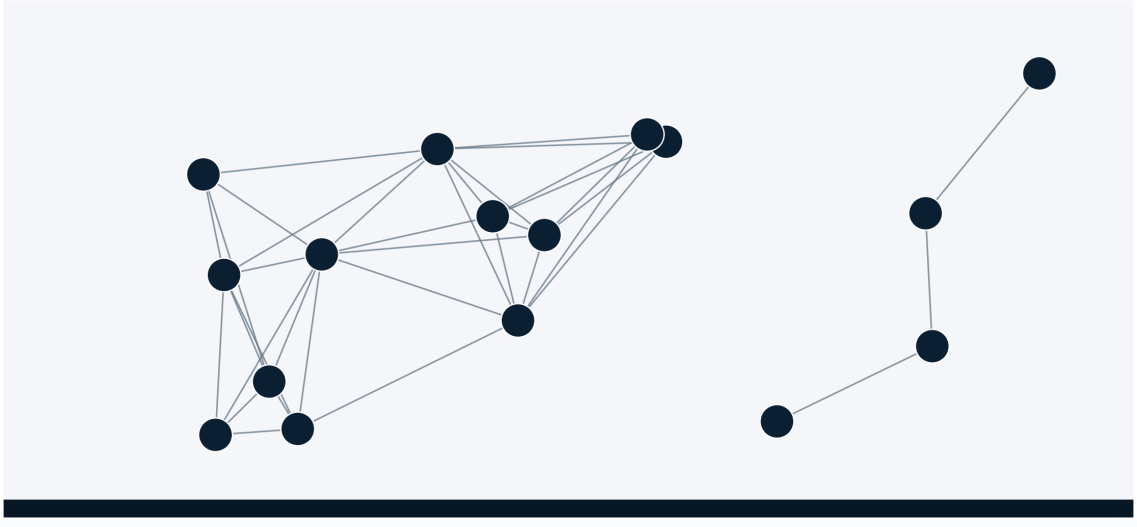
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Attachment Theory and Attachment-Based Therapy**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Attachment Theory and Attachment-Based Therapy to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 35

# Solution-Focused Brief Therapy and Strength-Based Practice

Scaling, exception questions, and the deliberate use of what is already working.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of the solution-focused school and the clinical problems that produced it.
- State the core assumptions of solution-focused practice and explain the clinical argument for each.
- Describe the five core skills: the miracle question, exception questions, the past question, the coping question, and the goal.
- Describe the scaling questions and explain their clinical use.
- Explain the concepts of the solution and the exception and their relationship to the problem.
- Apply the solution-focused approach to a case involving an adolescent and a family.
- Critically evaluate the cultural and ethical dimensions of solution-focused work, including the questions of language, of hope, and of the treatment of the problem.

## Chapter Overview

The solution-focused school began with a clinical observation that sounds trivial and is not. In the late 1970s, the family therapists in Milwaukee asked a family what had changed since their first appointment, and the answer was frequently: *nothing much*. The question was so ordinary that it reorganised the field, because it exposed the fact that the therapists had been working exclusively on the problem and had never asked what was working.

The approach that grew out of that question is a short-term, future-oriented, and deliberately non-exploratory therapy. Its core discipline is the **exception** — the times when the problem did not occur, or occurred less, or was less intense — and its argument is that the exceptions contain the solution, because the factors that are present in the exceptions and absent in the problem are the factors that produce the change.

The approach has become one of the most widely used in the world, and it is worth understanding why. The reasons are partly clinical and partly economic. It is short, which matters in services with waiting lists. It is compatible with a wide range of theories, so it can be used as a common language in a team. It does not require the client to have insight, which matters with young people. And it produces a specific, concrete, and time-limited goal, which is exactly what a family in crisis needs.

The approach's limits are equally real and worth stating at the outset. Its emphasis on the future can be experienced by some young people as a refusal to engage with a present that is intolerable, and the field's own term for this — **the tyranny of the positive** — was coined by a critic of the approach. Its language is relentlessly positive, and there are contexts in which positivity is a form of denial. And the evidence base, while good, is concentrated in specific populations and specific contexts.

This chapter presents the theory, the skills, the applications with young people, the evidence, and the critical analysis.

# 1. Historical Context

## 1.1 The Milwaukee brief therapy project

The brief therapy project at the University of Wisconsin–Milwaukee in the late 1970s was a research programme into the common factors in psychotherapy, and it was led by Steve de Shazer, whose background was in behaviour change and in communication theory.

Two features of the project were decisive. First, it was **empirical in a way that family therapy had not been**: the therapists systematically recorded the sessions, and they recorded the specific verbal and nonverbal behaviours of the therapists, and they analysed what the therapists actually did that distinguished the sessions in which clients improved. Second, the analysis found that the therapists' **questioning** accounted for a substantial proportion of the variance in outcome, and that the questions that predicted improvement were questions about the future, about the exceptions, and about the client's own resources.

The finding was not that the therapeutic relationship did not matter; the project's later work found that the relationship was a precondition. The finding was that, once the relationship was established, the specific behaviours of the therapist predicted the outcome, and that those behaviours were largely a matter of the questions asked.

### 1.2 The solution-focused brief therapy

The clinical practice that followed developed the model's techniques into the five core skills, and the practice was taught in a distinctive way: through a formal **training in listening**, in which the trainee learns to identify the client's own solution language as it appears in the conversation and to build the session around it. The training is unusually demanding and it is unusually effective.

The school's influence has been enormous and is best seen in the fact that solution-focused language is now used in services that have no connection to the school: in motivational interviewing, in coaching, in school counselling, in the management of chronic conditions, in the work with homelessness, and in the school-based prevention programmes of the 2000s.

### 1.3 The wider strength-based movement

Solution-focused practice is part of a larger movement that includes the strengths perspective in social work, the resilience literature in developmental science, the positive psychology that emerged in the 1990s, and the practice-based evidence movement. The shared proposition is that the questions asked of a client, a family, or a community determine the kind of information that the system produces, and that a system that is asked only about problems will produce only problems.

The proposition is not merely rhetorical. It has a technical consequence that a student should be able to state: **the questions a practitioner asks determine what the practitioner can see.** A practitioner who asks only about the problem will see the problem, and will become an expert on the problem, and will find that the problem is resistant, and will conclude that the client is resistant.

## 2. The Core Assumptions

### 2.1 The assumptions

The solution-focused approach rests on a small number of assumptions, and each of them has a clinical consequence.

**Every client has resources and strengths.** The assumption is not that the client is well or that the problem is not real; it is that the resources for the change exist somewhere, and the clinical task is to find them.

**Clients are the experts on their own lives.** The practitioner does not interpret, does not diagnose, and does not tell the client what is happening. The practitioner asks questions that help the client notice what they are already doing that works.

**Change has already happened.** This is the assumption that most distinguishes the approach and the one that most needs explanation. The client's life contains times when the problem was better, smaller, absent, or different, and these times are the raw material of the work. The exceptions are not a distraction from the problem; they are the solution in embryo.

**The future is the focus of the work.** The emphasis is on the next small step rather than on the last big one.

**The small can be a catalyst.** A small step that is achievable and that the client can verify produces a momentum that is disproportionate to its size, and the reason is that the small step is also evidence.

**Cooperation is the key.** The practitioner and the client work together on the problem, and the practitioner brings the question and the client brings the answer.

**The problem is separate from the person.** The person is not the problem, and this is the reframe that the approach offers to every client.

### 2.2 The solution and the exception

The **solution** is the behaviour, or the state of affairs, that the client wants more of. It is defined by the client, in the client's own words, and it may be entirely outside the problem: a young person who is being bullied may want to be "not frightened in the corridor", and that is the solution, and it is not the same as "not being bullied", and the distinction matters because the second may not be within the client's control.

The **exception** is a time when the problem did not occur, or occurred less severely, or was managed differently, and the exception is the search for the causal factors. The practitioner asks: what was different? What did you do? Who was there? What was happening in the other part of your life? The answers identify the factors that are present in the exception and absent in the problem.

**The exception is not the same as the solution.** The solution is what the client wants more of. The exception is a time when the client got some of it. The distinction is clinically important because the exception is evidence and the solution is the target, and a client who is asked to find the solution before the exceptions have been explored will often offer an aspiration rather than a description.

## 3. The Five Core Skills

### 3.1 The miracle question

**"Suppose that for some reason the problem got solved. What would be different?"** and then, if the client needs it, **"and what would you be doing?"** and **"and what would other people notice?"**

The miracle question is the signature technique of the approach, and its logic is worth understanding. It is a question about the future, it is deliberately large, and it is deliberately counterfactual. The counterfactual does the work: asking a young person to imagine the problem solved produces a description of the consequences of the solution, and the description is of a world in which the problem is not there, which is

information the client has and the practitioner does not.

**The therapist's discipline.** The therapist must resist the strong pull to ask the question in a way that suggests the answer, to ask a "realistic" version, or to move on to the next question. The miracle question works because it is asked as a genuine open question and because the client is allowed to answer it with something entirely impractical, and the impractical answer is often the most informative.

**With young people.** The miracle question is often better asked in the future tense and in the concrete: "If the thing with school got sorted out, what would your Tuesday look like?" The concrete form produces more usable detail.

## 3.2 Exception questions

Exception questions ask the client to describe a time when the problem did not happen, or happened less, or was handled better.

**"Was there a recent time when this was not a problem?"** and then, when the client has found one, **"What was different then?"** and **"What did you do that was different?"** and **"Who else was involved?"** and **"What was going on in the rest of your life at that time?"**

The exception questions have two functions. They **locate the solution** — because the factors present in the exception are the factors that make the difference. And they **disrupt the problem's hold** — because the problem's apparent all-encompassing quality is itself a distortion, and the exception is the data that corrects it.

**The therapist's discipline with exceptions.** The therapist must find a *recent* exception, and must resist the client's tendency to produce a very old one or one that is so distant that it is not usable. And the therapist must treat the exception as informative rather than as a lucky accident, because the treatment of the exception as an accident teaches the client that exceptions are rare and the search is hopeless.

**The exception and the small step.** The exceptions also identify the small steps, and the small steps are what the client does between sessions.

## 3.3 The past question

**"Was there a time when this was better than it is now? Or a time when it was even worse?"**

The past question serves a specific purpose: it locates the trend. A client who can identify that the problem has been improving slowly, however slightly, has a very different relationship with it from a client who experiences it as static or worsening, and the first client can be asked what has been helping.

The past question is also the technique for the young person who presents with a sense of hopelessness, because the question does not presume improvement: it asks about a time when it was *worse*, and the direction of travel is then established from the client's own material.

## 3.4 The coping question

**"What have you been doing to cope with this?"** and **"What is it that keeps you going?"** and **"How have you managed to keep going?"**

The coping question is the approach's answer to the question of resources, and it has a specific and important target: the coping that the client has developed is a *skill*, and the skill is usually already partly

generalised, and the therapist's task is to make it explicit and to extend it.

**With a young person who is in crisis, the coping question is often the first productive question in the entire session**, because it honours what they have done, and because the answer is usually more concrete than the description of the problem.

## 3.5 The goal

The **goal** is the specific, achievable, and time-limited target that the client sets, and the setting of a goal is the last and most important of the core skills.

**A well-formed goal** is: specific, so that the client will know when it has been achieved; achievable, so that the client believes it; and bounded in time, so that it can be checked.

The boundedness in time is the element most often missed, and it is what makes the goal a tool rather than a wish. "By the end of next week, I will have texted my cousin twice" is a well-formed goal. "I will be better" is not.

**The therapist's discipline.** The therapist does not set the goal, and does not improve it, and does not suggest a goal when the client is silent. A goal that the therapist has proposed and the client has accepted is not a solution-focused goal.

## 3.6 Scaling questions

The **scaling** technique asks the client to rate a dimension on a scale, and it is the most flexible of the school's techniques.

**"On a scale of zero to ten, where zero is the worst it has ever been and ten is the best you can imagine, where are you now?"** and then, **"And what would one point higher on the scale look like?"** and **"And what would one point lower look like?"**

The three questions are the whole technique, and each does something different. The **current rating** locates the client. The **"one point higher"** question identifies the immediate next step, and it is remarkable how much more specific a one-point shift is than the miracle question, because the one-point shift is within the client's own frame of reference for the scale and is therefore grounded in their own experience of the dimension. The **"one point lower"** question identifies what the client fears, and it is the question that surfaces the risk, and it is the question that the therapist should ask when the client is becoming complacent.

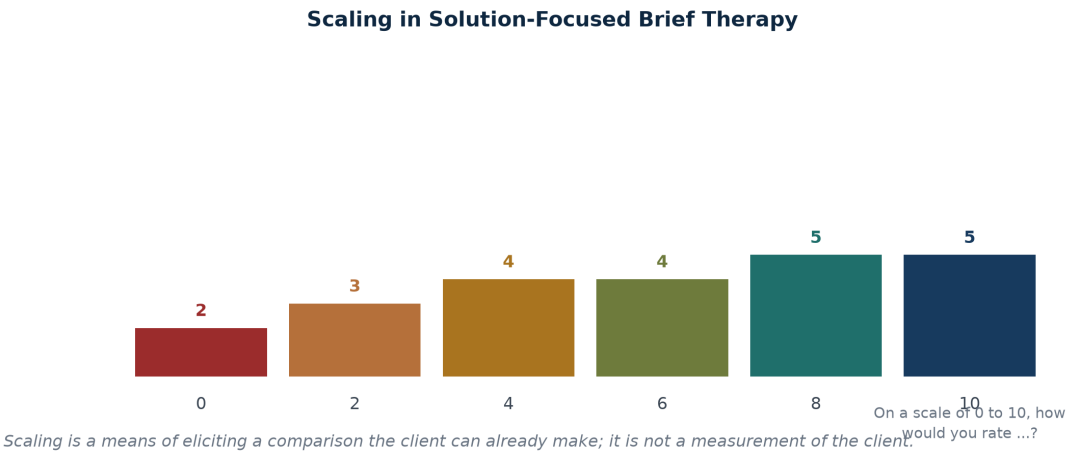



**Figure 35.25** The scaling question sequence, showing the current rating, the one point higher, and the one point lower, with the function of each in the session.

**Scaling in the session.** The scaling technique is also used on the *session* itself: "On a scale of zero to ten, how useful was that last ten minutes for you?" The practice of scaling the session and scaling the goal is one of the most distinctive and most immediately usable elements of the approach, and it is a good way for a student to begin practising the model.

## 3.7 Solution talk and problem talk

The most reliable single indicator of whether a conversation is solution-focused is the language it uses, and the distinction is worth learning as a listening skill.

**Problem talk** describes the problem in the third person, in the general, in the past, and in the abstract: *the thing with school*, *my son and his behaviour*, *I am always fighting with her*, *things have been really bad lately*. It is generalised, it excludes the client, and it contains no specific moment and no verifiable detail.

**Solution talk** describes a specific moment, in the present or the recent past, with the client in the first person, and with a detail that can be verified: *on Tuesday I got up at eight and caught the bus*, *last week I told my tutor I was coming back and then I left*, *I ate lunch in the canteen on Thursday and Finn was there*.

The practitioner's task in the conversation is to shift the talk from the first to the second, and the shift is accomplished not by telling the client to be more specific but by modelling the question that invites it. "Tell me about the problem with school" produces problem talk. "Tell me about the last time you went to school and it was okay" produces solution talk. "What did you do this morning?" produces solution talk.

Two practical notes follow. First, the shift is often achieved with a single word — the past tense for a future question, the first person for a third person. Second, and more importantly, the shift is a *social* process rather than a technical one: the client will begin to use solution talk when they experience that the practitioner is interested in the times things went better, and this is experienced as a change in what the practitioner wants, not as a change in the client's own thinking.

## 3.8 The practice in other settings

The solution-focused approach has been adopted more widely outside the therapy room than any other model in this book, and a student preparing for practice should know how it transfers.

**In schools.** The approach is used in school counselling with a striking consistency, and the reason is that it is quick, that it produces a written goal that the young person can take to the classroom, and that it does not require the young person to disclose. A school-based solution-focused session with a young person who is refusing to attend can be complete in fifteen minutes and can yield a goal that the form tutor can act on.

**In motivational interviewing.** The two approaches share a great deal: the focus on the client's own language, the focus on discrepancies between the stated values and the current behaviour, the resistance to telling the client what to do, and the eliciting of the client's own reasons for change. The scaling question and the "one point lower" question are structurally similar to the confidence ruler in motivational interviewing, and a practitioner who knows both will find the combination very powerful.

**In health and chronic illness.** The approach has been applied to diabetes management, to chronic pain, to medication adherence, and to weight management, and the clinical logic is the same: the exception search identifies what makes the behaviour possible, and the goal is a behaviour.

**In the justice system.** Solution-focused brief therapy has a substantial evidence base in community justice settings, and the reason is both clinical and practical: the clients are often hostile to the institution, they are often mandated, and the approach asks them no more than it needs to ask and produces a concrete action.

**In homelessness services.** The approach has produced some of the most encouraging results in the brief intervention literature, in a population that is typically difficult to engage and typically dropped out.

**With families in poverty.** The strengths-based practice literature, which grew out of the solution-focused work, has made a distinctive and important contribution: it has shown that the conventional deficit-based assessment systematically misdescribes families who are poor, because the conventional instruments ask about problems and the families have plenty, and the strengths-based instruments ask what is working and the families also have plenty, and the difference in the resulting assessment is a different family.

## 4. Further Reading for This Chapter

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- Fredrickson, B. L. (2013). *Positive emotions*. — The broader positive psychology.
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# 5. Solution-Focused Work with Children and Adolescents

## 5.1 The fit

The solution-focused approach is used more widely with young people than with any other population, and the reasons are worth understanding.

**It does not require insight.** A young person who cannot or will not articulate their difficulties can still answer the miracle question, the exception question, and the coping question, and the approach is therefore unusually accessible to a young person who is defensive, or inarticulate, or frightened, or who has been told by adults that their problem is bigger than they can manage.

**It is short.** A young person who has waited six weeks for an appointment and who suspects that the appointment will be another demand will engage with a short, concrete, future-oriented session in a way they will not engage with an open-ended exploration.

**It does not require the young person to be the problem.** The problem is separated from the person, and the young person is invited to be the expert on their own life, and this is a considerable relief to a young person who has been the identified patient in every previous attempt.

**It is compatible with the behavioural and the systemic work.** The exception question is, in the terms of Chapter 23, a question about the contingencies that were in operation; the small step is a graded task; the goal is a specification of a target behaviour with a time limit. The approaches fit together well.

**It produces something the young person can take away.** A written goal, a small step, a scale, and a telephone number are all things a young person can use between sessions, and a young person who is in trouble needs something to hold.

## 5.2 The specific modifications

**Use the child's language.** The approach's core discipline of using the client's own words is more important with young people than with adults, because the young person's words are also the material the young person will use outside the session. If a young person says the school is "a prison", the goal should be in the language of prisons, or at least should use the young person's words for the desired state.

**Ask the miracle question in the concrete.** "If the thing with school got sorted out, what would your Tuesday look like?" produces more usable detail than the abstract form, and it is easier for a young person to answer.

**Scale the small things.** The scale works particularly well with young people, and the "one point higher" question is the workhorse: "Where are you on a scale of zero to ten for how much you want to go to school?

What would one point higher look like?"

**Find the exception in the ordinary.** The exceptions with young people are frequently in the ordinary week: the good lesson, the day the tutor was away, the time the argument did not happen, the evening that was fine. The therapist's job is to get the granularity right.

**Use the coping question to establish the baseline of competence.** "How have you managed to keep going?" is often the first question that produces a competent description of the young person, and it changes the frame of the session.

**Set the goal with the young person and make it a behaviour.** A goal of "feeling better" cannot be checked. A goal of "texting my mum on Monday and Wednesday" can.

**Use the family.** The approach's techniques work with the whole family and they work particularly well with parents, because the coping question to a parent — "how have you managed to keep this together?" — is often the first time the parent has been asked anything other than what is wrong.

## 5.3 Solution-focused work with a family in crisis

A family in crisis is a good fit for the approach and a poor fit if it is applied naively. The specific cautions are as follows.

**Do not go too fast.** The "one small step" rule is right for a family in a stable situation and wrong for a family in which a young person is at risk. A family in crisis needs enough time in the session to be held, and the short session is a decision to be made deliberately rather than by default.

**Do not scale the safety.** Scaling the young person's suicidality, or the family's functioning, in the first session of a crisis is a misuse of the technique. The scale is for a dimension that the client can hold, and a scale on a crisis dimension produces a number that is meaningless and is experienced as a reduction of the crisis to a number.

**Use the exception to find what was there before the crisis.** The "what was different" question is the most useful question in a crisis, because the answer identifies what was working and has stopped working, and the work is often the restoration of something that has been lost rather than the creation of something new.

**The goal in a crisis should be a safety goal, and it should be co-constructed with the referrer.** The goal of not going to school is not a useful goal in a crisis; the goal of going to the school nurse's office at eleven, with an adult waiting, is.

## 5.4 The problem of the positive

The approach's language is relentlessly positive, and this is its most significant clinical liability. The phrase **the tyranny of the positive** was coined to describe the experience of a client who is invited to focus on their strengths at the moment when they most need to describe their pain, and who experiences the invitation as a demand that they perform wellness.

Three safeguards are available.

**Ask about the pain explicitly and in the same tone.** "I want to hear about the hard stuff too, and about the times when it is worse" is a legitimate and necessary question, and it is the safeguard.

**Do not imply that a difficulty is the client's responsibility.** The approach's language of "the problem is separate from the person" must be used carefully, because a young person who has been told that the

problem is theirs has had the framing imposed on them.

**Watch for the young person who is polite.** A young person who has learned to give the answer the adult wants will produce miracle answers and coping answers that are not true, and the practitioner's diagnostic task is to notice the discrepancy between the answer and the young person's presentation.

### 5.5 The problem of hope

The approach assumes that a future is available to be worked on, and the assumption fails in a number of circumstances that are common in the adolescent population.

**A young person who has no reason to want a future** — a young person who is suicidal, or who is in a family in which the future is genuinely uncertain — will produce answers to the miracle question that are either empty or despairing, and the practitioner must not treat the despairing answer as a failed miracle question.

**A young person for whom the goal has been confiscated.** A young person in a care placement whose every decision is made for them cannot set a goal, and the practitioner should work with what they can control, and should be honest with the system about the constraint.

**A young person whose future is genuinely not available** — where there is no realistic prospect of a stable home, or of safety, or of health — needs work that is not solution-focused, and the approach's optimism will be experienced as indifference if it is not matched by an honest assessment of the circumstances.

## 6. Case Illustration: The Young Woman Who Had Two Good Tuesdays

**Presentation.** Amira is seventeen and is referred by her school for "low mood and withdrawal". The referral describes a young woman who has stopped attending her college, who has stopped answering messages, who is "always in her room", and who has been "like this since October".

**The first session.** The practitioner does not begin with the problem. The practitioner begins with the coping question: "Before we talk about how things have been, I want to know how you've been managing. You've been through a lot this year — what's kept you going?"

Amira is quiet for a moment and then says, quite steadily: "Music. And my gran. And Tuesday." The practitioner asks what is special about Tuesday and Amira says that on Tuesdays she has a music lesson in town, and that she goes with a friend, and that she comes back and the evening is fine. The practitioner asks what happens on a Tuesday that does not happen on a Wednesday and Amira says: "Nothing happens. I just don't want to."

The practitioner asks a scaling question: "On a scale of zero to ten, where's your Thursday this week, and where's your Tuesday?" Amira says Thursday is a two and Tuesday is a seven. The practitioner asks what a three on Thursday would look like, and Amira says: "Getting out of bed before my dad shouts. And eating something that isn't in my room." The practitioner asks what a three would look like on the actual Thursday, and Amira says: "Get to college. Even for one lesson. And tell the tutor I'm coming."

**The exceptions.** The practitioner spends most of the session on the exceptions, and the exceptions are specific. On the Tuesday, Amira gets up at eight because she wants to catch the bus. On the Tuesday, she has something to wear that she has chosen. On the Tuesday, her phone is charged because she needs it for

her friend. On the Tuesday, her dad is working late, and her gran is there. The practitioner asks what else is different, and Amira says, after a pause: "I don't have to talk to anyone. In the tutor's office, I'm in a corridor and I don't have to speak."

**The formulation.** The formulation is not a depression formulation. The formulation is: Amira's week is structured by two things — a music lesson and the presence of her grandmother — and everything else is structureless, and the structureless days are the difficult ones. The withdrawal from the college is not a mood; it is the absence of the structure and the presence of the requirement to speak to people she does not know. Her mother, who is in the referral as a problem, is not in the formulation at all, and the practitioner notes that she has not yet heard from the mother, and she arranges to.

**The intervention.** Four elements.

*Element one: the goal.* Amira's goal, which she states herself, is: "By Friday, I go to the tutor's office and say I'm coming back, and I don't have to stay." The goal is specific, it is achievable, it is bounded in time, and it is a behaviour. The practitioner asks what she will do in the office and Amira says: "Stand at the door and say it. Then leave."

*Element two: the small steps.* The steps are derived from the exception, and they are the Tuesday factors applied to the difficult days. The first step is that Amira will get up and dress before her dad shouts, and the second is that she will charge her phone, and the third is that she will have something to wear that she has chosen. The purpose of the steps is stated explicitly: "You get dressed and charge your phone on a Tuesday because those are the things that make a Tuesday a Tuesday, and I'm asking you to try them on a Thursday, and if it works even a bit, that is evidence that a Thursday can be more like a Tuesday."

*Element one, revised: the grandmother.* The practitioner asks whether the grandmother could be involved and Amira says no, and the reason is that the grandmother is the person who looks after her and the family would be worried. The practitioner accepts this and does not pursue it, and the acceptance is itself an intervention, because it is the first time in the session that an adult has not pushed past a "no".

*Element three: the mother.* The practitioner sees the mother separately in the following week, and the mother's account is that Amira is "in her room all day, she's rude, she doesn't want to talk to me, she snapped at me on Sunday and I cried in the bathroom." The practitioner asks the mother the coping question — "How have you been managing?" — and the mother says, after a pause: "I don't know. I've just been going to work." The practitioner asks the mother to identify the last time she and Amira had a conversation that was not about a problem, and the mother cannot think of one. The practitioner asks the mother what Amira was like before October, and the mother says: "She was funny. She was really funny." And then she says: "I've been bringing her food up to the room. I thought that was helping."

The practitioner names the mechanism carefully: "I don't think you're doing anything wrong. But the food is the only contact you're having with her, and it's happening at her door, which means she doesn't have to come to you. And it's probably the only time you two talk. And if that's the only contact, then everything you want to talk to her about is going to go through that door, and she's going to close it."

The specific change: the food goes to the kitchen, and the mother and the daughter eat together once a week, on a fixed day, and the mother is asked to make one thing that is not on the ordinary rotation, and the conversation is not about the problem.

*Element four: the follow-up.* The session is booked for the following week, and the practitioner asks Amira to rate the session on the scale, and Amira says: "Seven." The practitioner asks what a seven-and-a-half would look like, and Amira says: "Texting my cousin. She keeps texting and I keep not replying and it's

starting to be weird." The practitioner takes this as the small step for the week, and it is the correct one, because it is the smallest social behaviour in the account.

**The outcome.** At six weeks, Amira is attending one lesson a week, has returned to the music lesson, and has replied to her cousin. Her mother reports that they eat together once a week and that "it's not brilliant but it's something". The college has not reported a full-time return, and the practitioner says clearly that this is not the outcome the referral wanted and is nevertheless the outcome that is durable.

**The interpretation.** The case illustrates the approach in full, and it also demonstrates its characteristic mechanism: the exception was located with precision, and the intervention was the transfer of the factors present in the exception to the days on which the problem occurred. The mother's daily delivery of food to the door was the sequence in the family system, and it was interrupted by the same logic, and the intervention with the mother was a single concrete change. And the therapist's discipline about the client's own words was what produced the formulation: the whole account was built on "Tuesday" and on what Tuesday contained, and no diagnostic formulation of a depressive disorder would have generated that intervention.

# 7. Evidence and Research Support

## 7.1 The status of the evidence

The solution-focused approach has a good evidence base, and it is stronger than its clinical reputation in some quarters would suggest.

**The brief therapy outcome literature is good.** The studies of brief therapy in general, in which the solution-focused approach is the most fully operationalised, show substantial effects on a range of outcomes and good retention.

**The research on the specific question types is the distinctive contribution.** The studies that recorded the therapists' questions and correlated them with outcomes found significant effects for the future-oriented and the exception questions, and the effects held after the therapeutic alliance was controlled for.

**The meta-analyses of the solution-focused approach report moderate-to-large effects** on depression, on the broad measures of psychological distress, and on the goal-attainment measures, and the effects are comparable with those of longer treatments in several populations.

**The evidence with adolescents is reasonable and growing**, and the approach is used extensively in school settings, and the school-based studies report good effects on the young people's measured outcomes and on the measures of school engagement.

**The evidence in specific high-risk populations is good.** The approach has been successfully applied in homelessness services, in the justice system, in chronic pain services, and in services for families with a young person at risk of removal from home, and the results in the homelessness literature in particular are among the most encouraging for a very brief intervention.

## 7.2 Limitations of the evidence

**The studies are small and heterogeneous**, and the model has been applied with considerable variation, and the fidelity measures are rarely reported.

**The measures are largely symptom and goal-attainment measures**, and the long-term outcomes are less well documented.

**The samples skew towards the mild and the voluntary**, and the young people in crisis are under-represented.

**The "positive outcome" findings are vulnerable to the response set of the participants.** A client who wishes to please a friendly practitioner who has asked them to describe a miracle will produce a miracle.

**The comparison with longer treatments is often unfair in either direction.** The studies frequently compare a brief solution-focused intervention with a longer treatment of a different theory, which confounds the brevity with the model.

# 8. Strengths and Limitations of the Solution-Focused Approach

## 8.1 Strengths

**It is accessible.** It requires no insight, no theory, and no trust in the therapist's expertise, and it works with young people who are resistant, inarticulate, or frightened.

**It is short and it retains people.** The approach has among the best retention rates in the psychotherapy literature, because people who are not helped do not come back.

**It is portable.** The five skills can be taught in an afternoon and used in any setting, and they are now used in services with no connection to the school.

**It is compatible with other modalities.** The exception question is a functional analysis in everyday language; the goal is a specification of a target behaviour; the small step is a graded task.

**It changes what the practitioner sees.** The question discipline changes the information the system produces, and a practitioner who has learned it becomes substantially better at seeing what is working.

**It is compatible with a working alliance.** The approach's posture of curiosity and respect is compatible with the person-centred conditions of Chapter 10, and the two are frequently combined.

## 8.2 Limitations

**It can trivialise.** The tyranny of the positive is a real and predictable harm, and a young person in pain can experience the approach as a demand that they perform recovery.

**It is not appropriate for every problem.** Where the problem is a danger, a symptom of a medical condition, or a consequence of a systemic failure, the focus on exceptions and goals is beside the point.

**It has no theory of aetiology, and this is a strength and a weakness.** It is a strength because it does not require a diagnostic formulation, and a weakness because the practitioner has no framework for understanding what they are seeing and no way of deciding what to do when the exceptions do not reveal a usable pattern.

**It can produce a compliant but unconfronted young person.** A young person who sets a modest goal, achieves it, and is congratulated has been given a success that is real and that may not change the underlying situation.

**The evidence does not address the severe cases**, and a service that adopts the approach wholesale will struggle with the young people it most needs to serve.

# 9. Cultural and Ethical Considerations

## 9.1 Culture and the language of the approach

The solution-focused approach's most powerful technique — using the client's own words — is also its most culturally sensitive, because the words are the culture.

**The miracle question assumes a future in which the problem is absent, and the future is culturally structured.** In a context in which the young person's future is materially uncertain, the question may be experienced as an accusation, and the answer may be despairing rather than evasive.

**"Strengths" are culturally specified.** The approach's assumption that the client's own language and the client's own people are the source of the strength is sound; the assumption that what the client values is what the client should work towards is not available, because the client's values are partly the values of a community that the practitioner may not understand, and in some contexts the young person's own stated goal will be a goal the practitioner regards as harmful.

**The exception search can be culturally loaded.** A young person in a family in which openness about the difficulty is itself dangerous may produce exceptions that are false, and the practitioner must be alert to the possibility that the exception is a performance.

**The language of the approach is individualistic**, and the "the problem is separate from the person" formulation is a claim about the person and the problem that does not transfer to every cultural frame, and in some settings the person and the problem are not separable in the way the formulation assumes.

## 9.2 Ethical considerations

**The obligation to acknowledge the pain.** A practitioner using this approach has an obligation to ask about the difficulty and to sit with the answer, and the approach's own literature now contains this caution.

**The obligation not to substitute positivity for safety.** A goal-setting process must not displace a safeguarding or a risk assessment.

**Informed consent to the approach itself.** A client should know what the approach does, and that it is brief, and that the practitioner is not going to explore the past in depth, and this should be said at the beginning rather than discovered at the end.

**The avoidance of coercion through goal-setting.** A goal that the client sets under the pressure of a therapist who is warm and encouraging and who has made the session pleasant is not a free choice, and the practitioner should notice the difference between a goal the client wanted and a goal the client produced to please.

**The equity issue.** The approach is culturally and economically attractive precisely because it is efficient, and a service under pressure may adopt it because it is cheap, and the risk is that it is then applied to the families who complain least. The approach works best with the families who can engage, and the families who cannot are the ones the service is under pressure to serve.

**The termination question.** A brief approach terminates quickly, and a young person who has experienced a short, warm, effective piece of work and then a termination may learn that relationships are brief, and the termination should be handled with the same care as any other.

## 10. Key Terms

**Key Terms at a Glance**

- **Solution-focused brief therapy** — A short-term, future-oriented, and non-exploratory therapy developed from the study of brief therapy in Milwaukee, in which the client's exceptions and resources are the primary clinical material.
- **The tyranny of the positive** — The experience of a client who is invited to focus on their strengths at the moment when they need to describe their pain, and who experiences the invitation as a demand to perform wellness.
- **Miracle question** — "Suppose that for some reason the problem got solved, what would be different?" The signature technique of the approach, in which the counterfactual produces a description of the consequences of the solution.
- **Exception question** — A question asking the client to describe a time when the problem did not occur, occurred less, or was handled better, and what was different about it.
- **Past question** — A question asking about a time when the problem was better or worse, used to locate the trend and to establish hope.
- **Coping question** — A question asking what the client has been doing to manage the problem, which identifies a skill that is already present and partly generalised.
- **The goal** — A specific, achievable, and time-bounded target set by the client, and the last of the core skills.
- **Scaling question** — A question asking the client to rate a dimension on a scale, followed by the "one point higher" and "one point lower" questions.
- **The one-point-higher question** — The question that identifies the immediate next step, grounded in the client's own frame of reference for the dimension.
- **The one-point-lower question** — The question that surfaces the client's fear and the risk, and the question to ask when a client becomes complacent.
- **Session scaling** — The practice of rating the usefulness of the session itself on a scale, and the most direct way for a beginner to learn the model.
- **Solution** — The behaviour or state of affairs that the client wants more of, defined by the client in the client's own words.
- **Exception** — A time when the problem did not occur, occurred less, or was managed differently, and the search for the factors present in it.
- **The distinction between the solution and the exception** — The solution is the target and the exception is the evidence, and a client asked for the solution before the exceptions have been explored will usually offer an aspiration.

**Key Terms at a Glance (continued)**

- **Solution language** — The client's own words for the desired state, which the practitioner uses verbatim and which is the primary material of the work.
- **The change has already happened** — The assumption that the client's life contains times when the problem was better, smaller, absent, or different, and that these times are the raw material of the work.
- **The small can be a catalyst** — The principle that an achievable and verifiable small step produces a momentum disproportionate to its size.
- **Problem is separate from the person** — The reframe the approach offers to every client, and one that must be used with care.
- **The questions determine the data** — The principle that the questions a practitioner asks determine what the system produces, and that a system asked only about problems will produce only problems.
- **The behavioural common factors** — The therapeutic alliance, the social contract, the expectancy of improvement, the client tasks, and the therapeutic feedback, and the approach's compatibility with the common factors model.
- **Cooperation** — The orientation in which the practitioner brings the question and the client brings the answer, in place of an expertise in which the practitioner brings the answer.
- **The small step** — The specific, doable action that the client takes between sessions, derived from the exception.
- **Question discipline** — The consistent use of the model's core questions, and the skill that separates a solution-focused practitioner from a generalist who asks the odd future-oriented question.
- **The training in listening** — The formal training in identifying the client's own solution language as it appears in the conversation.
- **Retain the client** — The principle that a client who is not helped will not return, and the approach's unusually good retention figures.

## 11. Chapter Summary

The solution-focused school began with the observation that therapists were working only on the problem and had never asked what was working, and it developed into one of the most widely used approaches in the world. The chapter traced the Milwaukee brief therapy project, in which the therapists' own recorded behaviour was analysed and the future-oriented and exception questions were found to predict outcome, and it placed the school within the wider strength-based movement and its shared technical proposition that the questions asked determine the information the system produces.

The core assumptions were stated with their clinical consequences, and the distinction between the solution and the exception was drawn: the solution is the target, the exception is the evidence, and a client asked for the solution before the exceptions have been explored will offer an aspiration.

The five core skills were described in detail, with the therapist's discipline for each: the miracle question, in which the counterfactual produces the description of the consequences of the solution and the practitioner must resist the urge to make the question realistic; the exception questions, which locate the solution and disrupt the problem's hold and must be recent and must be treated as informative rather than accidental; the past question, which locates the trend; the coping question, which identifies the skill; and the goal, with its three requirements of specificity, achievability, and boundedness in time, and the requirement that the client sets it. The scaling technique was described with the three questions that constitute it and with the one-point-higher and one-point-lower functions, and session scaling was identified as the most direct way for a beginner to learn the model.

The developmental and clinical fit with young people was developed through the five reasons the approach is widely used with this population, the six specific modifications for young people, the cautions for work with a family in crisis, and the two serious liabilities: the tyranny of the positive and the assumption of an available future. The three safeguards against the first and the circumstances in which the second fails were given.

The case of Amira demonstrated the approach's characteristic mechanism. The exception was located with precision — Tuesday, and everything Tuesday contained — and the intervention was the transfer of the factors present in the exception to the days on which the problem occurred, and the mother's daily delivery of food to the door was identified as the family sequence and interrupted by the same logic, and the whole formulation had been built out of the client's own words and would not have been produced by a diagnostic formulation of depression.

The evidence was summarised with its limits, and the strengths and limitations were set out, with the two most important limitations identified: that the approach can trivialise, and that it has no theory of aetiology, which is both its strength and its weakness.

The cultural analysis addressed the cultural structure of the future assumed by the miracle question, the cultural specificity of strengths, the possibility that the exception search can be culturally loaded, and the individualistic framing of the approach. The ethical section addressed the obligation to acknowledge the pain, the obligation not to substitute positivity for safety, informed consent to the approach, the avoidance of coercion through goal-setting, the equity issue that a service under pressure may adopt an efficient approach and apply it to the families who complain least, and the termination question.

## 12. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the clinical observation that produced the solution-focused school.
2. Describe the Milwaukee brief therapy project and its central methodological contribution.
3. Explain the technical proposition that the questions determine the data.
4. State the seven core assumptions and the clinical consequence of each.
5. Distinguish the solution from the exception and explain why the distinction matters.
6. State the miracle question and explain the role of the counterfactual.
7. State the therapist's discipline with the miracle question.
8. State the exception questions and explain their two functions.
9. State the therapist's discipline with the exceptions, including the recency requirement.
10. Explain the purpose of the past question and its use with a hopeless young person.
11. State the coping question and explain its value with a young person in crisis.
12. State the three requirements of a well-formed goal and explain the boundedness in time.
13. Explain the therapist's discipline in the goal-setting conversation.
14. State the three scaling questions and the function of each.
15. Explain the use of session scaling and its value for a beginning practitioner.
16. State the five reasons the approach is widely used with young people.
17. State the six specific modifications for young people.
18. State the three cautions for work with a family in crisis.
19. Explain the tyranny of the positive and the three safeguards against it.
20. Explain the assumption of an available future and the three circumstances in which it fails.
21. Describe the case of Amira and state the mechanism it demonstrates.
22. Summarise the status of the evidence for the approach.
23. State the six strengths and the six limitations of the approach.
24. Explain the four cultural considerations for solution-focused work.
25. State the seven ethical requirements for this work.

## 13. Practice Activities

**Practice and Application**

### Activity 35.1 — The Five Questions (60 minutes, individual)

Write out and answer, in writing, all five of the core questions for a problem of your own, and then write the answers as a client might give them. Then write the miracle question as a therapist tends to ask it by mistake, and compare. The exercise is the fastest way to learn the model's discipline, and the comparison of the two forms of the miracle question makes the difference between the real and the implied question visible.

### Activity 35.2 — The Exception Search (90 minutes, field-based)

Take a young person from placement and find three exceptions — times when the difficulty was smaller, or absent, or handled better. For each, write the "what was different" analysis: the specific factors, the people, the time of day, the state of the other part of the life. Then write the small step for each. Most students find that the exceptions are ordinary and that the factors are mundane — a charged phone, a chosen outfit, a friend present, a door open — and the discovery that the treatment is the transfer of the mundane is the point.

### Activity 35.3 — Scaling the Session (three sessions, in pairs)

In pairs, run a fifteen-minute conversation in which the practitioner asks only the five core questions, and at the end scales the session and asks the "one point higher" question. Then reverse roles, with the second student now acting as the practitioner, who should ask fewer questions and listen more. The second round is the instructive one, because it demonstrates the difference between a practitioner who is asking questions and a practitioner who is listening for the answer.

### Activity 35.4 — Auditing a Miracle Question (60 minutes, small group)

In groups of three, take a real referral and write the miracle question as it would be asked in that case, in the exact words the client would use, followed by the scaling question and the exception question. Then have each group member read the set aloud and identify the question that is most likely to produce a useful answer, and the one that is most likely to produce a polite one. Then write the version of the miracle question that would work with a young person who has been in trouble with adults before. The final step is the practical lesson of the chapter.

## 14. Annotated Reading Pathways

**For the model.** Read de Shazer (1985) and then O'Connell (2003), which is the clearest clinical handbook and the most usable with young people.

**For the research.** Read the original Milwaukee project papers and the later research on the question types, and read Shonk and Greasley (2015) for the contemporary account. The question-type research is the most interesting evidence in the book, because it is the evidence for a specific technique rather than for a treatment package.

**For the practice.** Read O'Connor and McGee (2016), which is written for parents and educators and is exceptionally clear about the mechanics. Then practise the five questions, because the model is a skill and not a set of propositions.

**For the evidence.** Read the meta-analyses of solution-focused therapy and of brief therapy, and the school-based studies. Read the criticisms alongside them.

**For the critique.** Read the literature on the tyranny of the positive, and read the critical accounts of brief therapy in complex cases. The critique is important because the approach's virtues and its failures are the

same feature seen from two sides.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Solution-Focused Brief Therapy and Strength-Based Practice** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is scaling, exception questions, and the deliberate use of what is already working. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Ines is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Ines's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Ines explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Ines chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Ines has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Ines can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Solution-Focused Brief Therapy and Strength-Based Practice

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Solution-Focused Brief Therapy and Strength-Based Practice** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Solution-Focused Brief Therapy and Strength-Based Practice** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Solution-Focused Brief Therapy and Strength-Based Practice**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a

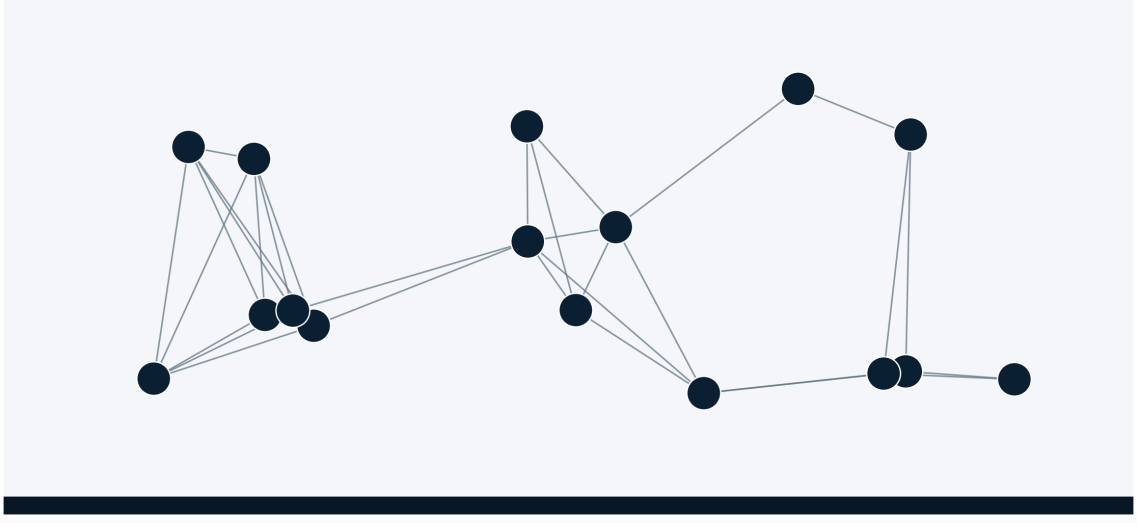
discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Solution-Focused Brief Therapy and Strength-Based Practice to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 36

# Narrative Therapy and the Re-Authoring of Lives

Externalisation, unique outcomes, and the collaborative reconstruction of adolescent identity.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of the narrative therapy movement and describe the intellectual context in which it emerged.
- State the core concepts of narrative therapy, including the idea of the person as the author of their own life, the externalisation of the problem, and the re-authoring.
- Describe the techniques of externalisation, the naming of the problem, the mapping of its influence and its influence on the young person, the unique outcomes, and the re-authoring.
- Explain the use of documents, letters, and the therapeutic group in narrative practice.
- Apply the narrative framework to a case involving an adolescent and a family.
- Critically evaluate the cultural and ethical dimensions of narrative work, including the questions of whose story is told, of power, and of the relationship between the personal narrative and the material circumstances.

## Chapter Overview

Narrative therapy is the most recent of the family therapies in this book and the one with the most substantial intellectual debt to the wider human and social sciences. It came out of the late 1980s in Australia and New Zealand, and it was shaped by three sources that practitioners in the helping professions rarely combine: literary theory, particularly the structuralist and post-structuralist analysis of narrative; feminist epistemology, particularly the critique of the way in which knowledge about women has been produced by the professions; and the intellectual traditions of post-colonialism and of the social sciences' interest in how identities are constituted by discourse.

Its central claim is that **a person's identity is constituted by the stories they live by and tell about themselves, and that the stories are not the person**. This sounds abstract and is in fact highly practical. A young person who has lived for two years with the story "I am the one who ruins everything" has organised their behaviour around that story, their relationships around it, and their expectations about the future around it. The story is not a description of the person. The story is a framework through which the person has made sense of a life that has been genuinely hard, and the framework is now the thing that is producing the outcomes.

The practice of narrative therapy is therefore the practice of helping a young person to identify the stories that are organising their life, to examine them, to find the exceptions to them, to develop what the work calls **unique outcomes** — the moments and the events that do not fit the dominant story — and to use those unique outcomes to begin to author a different account of their life.

The approach is the most explicitly political of the therapies in this book, in the sense that it is concerned with how power operates in the production of knowledge, and this is both its greatest contribution and the source of its most substantial difficulties, which are addressed in Sections 9 and 10.

## 1. Historical Context

### 1.1 The development

The movement grew out of the family therapy practices of Michael White and David Epston in Melbourne and Auckland, from the mid-1980s. Both were family therapists with a social work and a literary background, and both were frustrated by two features of the family therapy of the period: its assumption that the family was a self-contained system that could be understood by mapping it, and its silence about the larger social structures within which families lived and about the power relations within them.

The break with the family therapy tradition was sharp. The structural and strategic schools treated the family as a system whose patterns could be mapped, and the narrative therapists asked a prior question: **by what criteria is this family understood to be a family at all?** A family that includes a violent father and an abused child is not a family in the way that the systemic models assume, and the framework of the family is itself a product of a moral and political judgement.

### 1.2 The intellectual sources

**The literary and post-structuralist analysis of narrative.** The structuralist narratologies of Propp, Todorov, and Genette, and the post-structuralist work of Barthes, Derrida, and Ricoeur, provided the analytic vocabulary: the distinction between the story and the discourse; the concept of the **fabula** as the raw material of lived events and the **syuzhet** as the organising shape given to them; the concept of the **narrative frame**, which selects certain events and excludes others; and the concept of the **absent or excised text**, the parts of the story that the frame excludes.

**Feminist epistemology.** The critique of the professional claim to know the truth about a client, particularly a woman, and the analysis of the ways in which a professional discourse can constitute the identity of the person it describes, came from the feminist critique of the therapies and the feminist research tradition.

**Post-colonial and social constructionist thought.** The analysis of how identities are constituted within discourses of culture, race, gender, and class, and the concept of the "othering" of people by the dominant discourse, informed the practice's treatment of the problem as an agent.

**Social work practice research.** The practice-based research movement in social work, of which the Australian and New Zealand family therapy tradition was a part, contributed the emphasis on the small, the local, and the specific.

### 1.3 The reception

Narrative therapy has been adopted in a very large number of settings, and it is used in services that have no connection to family therapy, in schools, in community mental health, in social services, in services for refugees, and in services for domestic violence. Its influence extends well beyond the practice of the model: the concept of the externalisation is used in coaching, in education, in organisational change, and in the self-help literature, and the **therapeutic conversation** in its modern form is discussed in Chapter 58 of this book.

## 2. The Core Concepts

### 2.1 The person as the author of their life

The foundational claim is that **a person is the author of their life**, and the word "author" is doing specific work. An author does not write a book in a single stroke; an author writes, revises, and re-writes, and a book

is the product of many drafts. The claim is therefore not that the young person has complete control over what happens to them, and the practice is careful to say so, but that the young person has a role in the shaping of the account of their own life, and that the account is a *draft* rather than a sentence.

Three corollaries follow.

**Identity is not fixed.** If the identity is constituted by the story, and the story is a draft, then the identity is revisable.

**The problem is not the person.** The problem is a thing that does things to the person, and it is the subject of a story, and it has interests of its own.

**Meaning is made, not found.** The meaning of an experience is not in the experience; it is in the account given of it, and the account can be different.

## 2.2 Externalisation

**Externalisation** is the practice of separating the person from the problem, so that the problem becomes an object that can be seen, described, named, and examined, and the person becomes the subject of their own life rather than the object of the problem's.

The classic example in the literature is the young person who says of their drinking, "The Drink took over my life." The externalisation takes the sentence and changes the grammar: "the drink" is given the attributes of an agent — it is powerful, it is persuasive, it takes over, it is cunning — and the young person is given the attributes of a subject who is acted upon and who can resist.

**The practice of externalisation** has a technical vocabulary, and the practitioner should be able to name it.

**Naming the problem or naming it differently.** The name is the young person's own, and the practitioner sometimes suggests a name and sometimes asks. "The Worry" and "The Fog" and "The Voice" are names that appear in the literature, and they are usually funny, and the humour is a feature: it is what makes the problem nameable without it being an insult.

**Writing the problem's influence.** "What does the worry do to you? What does it make you do?" and the answer is written down, often by the young person themselves, and often in the form of a list.

**Writing the problem's influence on the young person's influence on others.** "What does the worry make you do to your mum? What does it make you do to your friends?" The second question is the crucial one, because it locates the problem in the young person's life as an agent with power, and it makes the young person's own actions explicable.

**Identifying the unique outcomes.** "Are there any times when the worry doesn't have as much power? What did you do to make that happen?" and this is the heart of the practice.

**Authoring a new story.** "If the worry is not in charge, what would your life look like? How would your friends know? What would be different at school?" and the new account is written.

## 2.3 The problem's influence and the young person's influence

The **problem's influence** is the network of effects that the problem has on the young person's life: what it prevents, what it makes them do, what it costs them. The **young person's influence** is the network of effects that the young person has on their own life and on others: what they do that makes things better, what they are good at, what they influence in other people.

The practice of mapping both is the most structured technique in the model, and it is usually done on paper: a diagram with the problem at the centre, arrows going out to the effects it has, and arrows going in from the areas of the young person's life in which they have influence.

The clinical significance is that the second map is almost always thinner than the first, and this is the point. The young person has been living in a world in which the problem is the most powerful thing in it, and the discovery that they already have influence in some areas — however small — is the beginning of a different account.

## 2.4 Unique outcomes

A **unique outcome** is an event, a moment, a feeling, a thought, or a piece of learning in the young person's life that does not fit the dominant story, and that therefore opens a space for a different story.

The criteria are specific, and the practitioner should be able to state them.

**The unique outcome must be the young person's own.** It is not something the practitioner suggests and it is not something that happened to them.

**It must be a specific event, not a generalisation.** "I was able to tell my dad I was not going to the party" is a unique outcome. "I am a more assertive person" is not.

**It must be a departure from the dominant story.** If it fits the story, it is not a unique outcome, and a practitioner who cannot find one has usually asked the wrong question.

**It must be sufficiently detailed to be re-told.** The detail is what makes the re-authoring possible, because the re-authoring works by re-describing the same event with a different meaning.

**It must be linked to a skill or a preference or a value that the young person already has.** The unique outcome is not a fantasy; it is evidence of a capacity that already exists.

## 2.5 The dominant story and its problems

The **dominant story** — in the older literature, the "problem-saturated story" — is the account of the self that the young person lives by, and the practice of naming it is a distinctive and important step.

A dominant story is characterised by: a **subject position**, which is the self that the story requires the young person to occupy, usually a self that is the cause of the problem; a **plot**, which is the sequence that the story tells; and a **moral**, which is the conclusion the story draws about the young person and about the world.

The dominant story of a young person referred for behavioural problems in school frequently runs something like: "I am the one who causes trouble, and trouble follows me everywhere, and there is nothing to be done about it." The story is coherent, it is widely reinforced by the responses of others, and it is self-fulfilling: a young person who is the one who causes trouble behaves in the way that confirms the story, and the confirmation is read as evidence for the story.

**Naming the story** is done carefully and with permission. The practitioner asks: "How would you describe someone in your situation? What would you say is the story about you?" and the young person's own words are used, and the question "What is the effect of this story on your life?" opens the examination.

## 2.6 The re-authoring

The **re-authoring** is the process of rewriting the account of the young person's life so that it includes the unique outcomes, and it is the outcome of the whole practice.

The re-authoring is not the same as the positive reframe of the solution-focused school, and the difference is worth stating. A positive reframe replaces a negative account with a positive one. A re-authoring expands the account, and the expansion makes room for the unique outcomes, and it does not require the negative account to have been false. The young person who has been bullied did not imagine it, and the re-authoring does not require them to say otherwise; it requires them to be able to say that the bullying did not determine the rest of their life, and to point to the specific evidence.

The technique of the re-authoring is to return to a specific unique outcome and to re-describe it, in detail, with the young person dictating, in their own words, and then to ask the questions that extend the account: "How does this change your understanding of what happened? If this is who you are in some situations, what does that mean for the other situations? Who else would have to change their story to make room for this?"

The last question is the one that makes the work systemic, and it is the question that distinguishes narrative therapy from a purely individual practice.

# 3. The Techniques in Detail

## 3.1 Externalising conversations

**The grammar of externalisation.** The work is done in the language. "The problem makes you do" becomes "You do" and "the problem" becomes the subject. "The Worry" is given a name, a personality, a set of characteristics, and an agenda. A young person who says "the worry takes over my body" is invited to describe what the worry does, and the describing is the intervention, because the young person is now observing the worry rather than being inside it.

**The questions that externalise.** "What does the problem do to you?" "What does the problem want?" "How does the problem talk to you?" "What would the problem say if it could speak for itself?" "What does the problem do to make sure you do what it wants?" These questions are the technique, and they are asked slowly, and they are asked in a tone of genuine curiosity.

**The rules of externalising questions.** They are asked of the problem and not of the person. They are asked more than once, because the first answer is usually a summary. They never suggest the answer. And they are not asked in a way that triviallyises: the question "what does the problem want?" presupposes that the problem wants something, and for a young person who has been told that their problem is their own fault, the presupposition is the intervention.

## 3.2 The documents

The **documents** are the most distinctive feature of the narrative practice, and they are of three kinds.

**The letter from the problem.** A letter written by the problem to the young person, in the problem's voice, describing what it has done to them and what it wants. It is usually written by the young person, sometimes dictated to the practitioner, and it is often the first document because it is the most engaging and the least threatening.

**The letter to the problem.** A letter written by the young person to the problem, often as a reply, in which the young person tells the problem what it may and may not do from now on. This is the beginning of the boundary-setting, and the letter is often more useful than a conversation because it is in the young person's own words and it can be re-read.

**The letter to the young person.** A letter from the practitioner to the young person, written at the end of a piece of work and given to them, and the letter is a record of what the practitioner has noticed and what the practitioner values, and it is often the artefact that the young person keeps.

**The "take-away" letter.** The most common of the documents: a letter written by the young person to themselves, to be read at a difficult moment, containing the unique outcomes and the re-authored account. Its efficacy depends entirely on the young person's own words.

**The value of the documents.** They externalise the conversation from the session; they slow it down, because writing is slower than speaking; they give the young person something to hold; and they make the re-authoring concrete, because the re-authored account has to be written in sentences.

### 3.3 The therapeutic group

The **therapeutic group** — also called the storytelling group — is the group in which the re-authoring is done collectively, and it is a distinctive and powerful element of the practice.

The structure is: each member tells their story to the group in a structured way; the group listens for the unique outcomes; and the group members then ask questions, and the questions are restricted to two forms — "what was different?" and "how come?" — with the rule that no advice, no interpretation, and no "have you tried" is permitted.

The rules are the therapeutic mechanism, and they are the point of the whole technique. The questions "what was different?" and "how come?" are questions that the young person answers, and the answers are the unique outcomes, and the group is a machine for producing them.

**The evidence for the group format is good**, and the group is used with adolescents, with families, and with adults, and it is the format in which the approach is most clearly distinct from the individual work of the other therapies in this book.

### 3.4 The role of documents in the school setting

In a school, the documents have a specific and useful function: they can be shared with the teachers. A letter from the problem, written by the young person, that describes what the problem does in the classroom and what it wants, can be given to a form tutor, and it converts the teacher's response from a reaction to the young person into a response to the problem. This is a small intervention with a large effect, and it is one of the most practically useful applications of the model in a school setting.

## 4. Further Reading for This Chapter

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# 5. Narrative Work with Children and Adolescents

## 5.1 The developmental fit

Narrative therapy is used extensively with young people, and the reasons are developmental as well as practical.

**Adolescence is the period of the identity narrative.** The central developmental task of the period is the construction of a coherent account of who the young person is and who they are becoming, and the accounts available in the surrounding discourse — from the family, from the school, from the peers, from the media — are in competition with the one the young person is building.

**The dominant story is often assigned from outside.** A young person who is in trouble, who is unwell, or who is in a family that is under strain is the recipient of a story that is spoken about them rather than by them, and the developmental risk is that they will adopt it.

**The adolescent's capacity for narrative is fully developed.** The ability to hold a complex account of one's own life, to see that there are different ways of telling the same events, and to recognise that the account is a construction rather than a fact, is a late adolescent capacity, and the practice is appropriate to it.

**The group's format is developmentally suited**, because adolescents are highly attentive to the questions asked by their peers and highly sensitive to the possibility of being told what to do.

## 5.2 The initial conversation

The narrative first session is distinctive, and a student should know what it involves.

**The practitioner tells the young person what the practice does.** The explanation is direct: "I am not going to ask you why you do the thing you do. I am going to ask you what the thing does to you, and what you do to it, and I am going to help you write down the parts of your life that the thing has not managed to reach. And at the end we are going to write a new version of your story that includes those parts." The young people who stay with the practice are frequently the ones who have never been told that the problem is not their fault.

**The practitioner asks the young person to describe the problem in their own words**, and does not supply a label. "What do the adults at school call it? What do you call it?" The name the young person chooses is usually more vivid and more externalising than the diagnostic one, and the use of the young person's own name is the beginning of the externalisation.

**The practitioner locates the problem's network.** "Where does this thing show up in your life? What does it do to your sleep? To your friends? To what you do at the weekend?" and the answers are written down, and the writing is the map of the problem's influence.

**The practitioner looks for the unique outcomes from the first session**, and the first session should end with at least one, however small, and the therapist should not leave the session without one, because the young person's experience of the session will be determined by whether the practice delivered what it promised.

## 5.3 Working with the school and the family

**With the school.** The letter from the problem is the most useful document in a school setting, and the practitioner should explain to the young person how it works before it is written, so that the young person can decide whether they want it shared. A letter from the young person that says "The Panic makes me do the breathing and it tells me everyone is watching and it makes me go to the toilet and it makes me get sent out of class" gives the teachers something to work with that is not the young person.

**With the family.** The narrative approach is unusually well suited to family work, because the practice of the family is to find the dominant stories that circulate in the family and to examine them, and the re-authoring is a collective act. The practitioner can map the family's dominant stories — "we're the family where nothing works", "she's the one who was always the difficult one", "we don't talk about that" — and the unique outcomes can be located in the family's own history.

**The systemic re-authoring question.** "Who else would have to change their story to make room for this?" is the question that connects the individual work to the family and to the school, and it is the question that a practitioner who is working systemically should always ask.

## 5.4 The work with the unique outcomes

The process of developing a unique outcome requires patience, and a student should know the common obstacles.

**The young person cannot find one.** This is common in the early sessions, and the usual reason is that the question is too big. The remedy is to reduce the time scale: not "a time when the anxiety didn't happen" but "a moment in the last week when you felt more like yourself", and the smaller the frame the more likely the

young person is to find something.

**The young person finds only negative unique outcomes.** A young person who says "the only time it was better was when I was on drugs" has found a genuine unique outcome, and the practitioner's task is to find the *components* of that experience. "What did you like about being on drugs? What did it give you that you wanted?" and the answer — the relief, the numbness, the confidence, the not-caring — is then located in other parts of the young person's life where the same thing can be had differently. This is the technique that the literature calls the **re-authoring of the unique outcome** and it is one of the most useful and least discussed.

**The young person produces a unique outcome that is too small.** "I was okay for about ten minutes" is a valid unique outcome and it is frequently dismissed by the practitioner, and it should not be, because the ten minutes can be examined for the factors present in it and the factors can be extended.

**The young person resists the re-authoring.** Some young people are invested in the dominant story, because it is a story in which they are the protagonist and it has a certain dignity, and the practitioner should note that the dominant story usually protects the young person from certain kinds of responsibility and from certain kinds of hope, and the examination should be gentle.

### 5.5 The limits with young people

**Not every young person can or will work this way.** The practice requires a capacity for reflection and a tolerance for ambiguity, and a young person in acute crisis, or with a very limited cognitive capacity, or with a very short attention span, may not be able to.

**The re-authoring can become a demand.** A young person who has been told that they must find their unique outcomes and who cannot, has been given another task they will fail, in a practice that promised them that they were the author.

**The problem is sometimes real and does not need externalising.** A young person who is being abused needs the abuse stopped, and the externalisation of "the abuse" into a separate agent, however therapeutically elegant, can be heard as a dilution of the reality.

## 6. Case Illustration: The Young Woman Who Had Become the Problem

**Presentation.** Keziah is sixteen and has been excluded from two schools in the past year. The referral is from her current school, which is worried about her attendance and about a fight in the corridor. Keziah's mother attends the first appointment and says, before the practitioner has said anything, "It's the same as it was with her father."

**The first individual session.** The practitioner does not begin with the history. The practitioner begins with the question: "What do the people at school call it? And what do you call it?" Keziah says: "They call me a behaviour problem. I call it the Black Hole." The practitioner asks what the Black Hole does, and Keziah says: "It eats everything. It ate my friends. It ate my year. It comes in at nine and it's there all day and it eats everything I try to do." The practitioner writes this down, and Keziah watches the words being written, and the practitioner says: "Can we keep going? What else does it eat?"

Keziah says: "It ate my music. It ate going to see my aunt. It ate my grades, but honestly the grades were gone before the Black Hole." The practitioner asks whether there is anything it has not eaten, and Keziah

thinks and says: "It hasn't eaten my swimming."

**The unique outcome.** The practitioner develops the swimming. "Tell me about the swimming." Keziah talks for twenty minutes without interruption. She swims four times a week, before school, at a pool her aunt drives her to, and the times are the best parts of her week. The practitioner asks what is different about the swimming, and the answers are precise: she is in the water and nobody can talk to her; she is doing a thing she is good at; she is with a coach who tells her what to do and does not have a mood about it; and her phone is in her bag.

**The mother's account.** In the joint session, the mother says that Keziah has always been "a handful" and that she was exactly like her father, and the practitioner asks the mother, gently, to say more about the father. The mother says that he was "a lovely man until he drank" and that he left when Keziah was three, and that she has spent thirteen years being careful that Keziah does not become him. The practitioner does not pursue this, but the practitioner does ask one question: "What did Keziah look like at three?" and the mother says: "She was the funniest child. Absolutely." And she stops, because she has heard it.

**The dominant story.** The practitioner names it explicitly in the session with Keziah: "So the story is that you are going to end up like your dad, and everything you touch turns into the Black Hole, and there is nothing you can do about it. And you have been living inside that story since you were three." Keziah says: "That's my mum's story. That's not mine." The practitioner asks: "So what is your story?" and Keziah says: "My story is that I am the person who is in trouble." And then she says, after a pause: "And that other people decide what that means."

**The re-authoring.** The re-authoring is done with a document. The practitioner asks Keziah to write a letter to the Black Hole, and suggests the first line: "I know what you want." Keziah writes, and the letter is three paragraphs long, and the third paragraph is: "You are not allowed in the pool. You are not allowed at my aunt's. If you want to be fed, feed me at breakfast, not at nine o'clock at night." The practitioner reads it and asks: "Are you sure this is what you want to send?" and Keziah says: "Yes."

**The systemic work.** The letter is shared with the mother, with the form tutor, and with the swimming coach. The form tutor's response is the most striking thing in the case: the teacher, who had been receiving the letter as an account of a difficult pupil, writes back a note saying that the Black Hole "sounds exhausting" and that the pupil is "not the problem, then". The mother writes back to Keziah a note that is in the third person — she says that she has realised she has been writing Keziah's story for her — and it is the first time the two have spoken in six months.

**The outcome.** At three months Keziah is in one school, is attending four days a week, is still swimming four times a week, and is in contact with her mother. The fight in the corridor has not recurred. The school has not made any formal change to her plan, which the practitioner regards as a missed opportunity, and the practitioner has a further four sessions and uses them to work with the mother and with the school on the question of the expectations.

**The interpretation.** The case illustrates the model's distinctive operations. The problem was named by the young person, and the naming was an externalisation; the unique outcome was located with precision and its components were identified; the dominant story was named, and the naming revealed that it was a story written *about* Keziah rather than by her, and the young person's own correction — "and that other people decide what that means" — was the pivot of the case. The re-authoring was done in the young person's own written words, and the document was the mechanism of the change. And the systemic question — who else has to change their story — was answered productively, because the teacher and the mother each had a story

about Keziah that they had never examined, and the letter from the problem gave them the material.

# 7. Evidence and Research Support

## 7.1 The status of the evidence

The evidence for narrative therapy is good in a specific and unusual way, and a student should be able to describe it accurately.

**The outcome research is the strongest in the family therapy field.** The studies that have used standardised outcome measures, and the practice-based research that has used single-case designs, have produced consistent findings of substantial improvement, and the effect sizes reported in the reviews are among the largest in the family therapy literature.

**The research on the distinctive outcomes is the most interesting contribution.** The studies of the *distinctive outcomes* — the specific changes in the young person's own language and in their own account that the research identifies as the markers of a successful narrative therapy — have identified a remarkably consistent set: the young person begins to use the problem's name less and their own attributes more; the unique outcomes become more detailed and more numerous; the young person begins to attribute change to their own actions; and the language of the dominant story shifts.

**The research on the group format is good**, and the studies of the storytelling group have found substantial effects and good retention.

**The evidence with young people is substantial**, across schools, community mental health, and family settings.

**The evidence base has been criticised on methodological grounds**, and the criticisms are worth knowing: the trials are frequently small, the fidelity of the delivery is difficult to measure and is rarely reported, and the practitioners who conduct them are often the developers of the model.

## 7.2 Limitations of the evidence

**The measures of the practice's distinctive mechanism are retrospective and interpretive**, and the identification of a "distinctive outcome" depends substantially on the researcher's judgement.

**The trials are few and small**, and the confidence intervals are wide.

**The generalisability to other cultures and other languages is uncertain**, given the practice's roots in a specific intellectual context.

**The evidence does not address the complex presentations**, and the practice with young people with severe mental health needs, with neurodevelopmental conditions, or with acute risk is largely untested.

# 8. Strengths and Limitations of Narrative Therapy

## 8.1 Strengths

**It is a theory of identity that is developmentally apt.** The account of the young person as an author of their own account is a better fit for the adolescent than most of the theories in this book.

**It locates the problem outside the person**, and it does so in a way that a young person can feel.

**It produces artefacts.** The letters and the documents are tangible, they belong to the young person, and they often outlast the therapy.

**It is compatible with systemic and family work**, and the re-authoring of a family story is a natural extension of the individual practice.

**The group format is efficient and powerful**, and it is unusual in producing a real sense of a collective practice.

**It gives the practitioner something to do when nothing is working.** The externalising questions work in almost any circumstance, and they work when the young person will not talk about the problem.

**It is explicitly attentive to power**, and it has produced some of the clearest ethical writing in the therapy field.

### 8.2 Limitations

**It can be used to individualise a structural problem.** This is the most serious criticism and it is discussed at length in Section 10.

**It requires a capacity for reflection that some young people do not have**, and the practice has relatively little to offer the young person in acute crisis.

**It is difficult to learn well.** The externalising questions are easy to learn and easy to misuse, and the practice's discipline lies in the quality of the listening, which is not teachable by a manual.

**Its conceptual foundations can be opaque in practice**, and the philosophical apparatus can obscure what the practitioner is actually doing.

**The measures of the practice are weak**, and the field has invested more in the articulation of its theory than in the evaluation of its outcomes.

## 9. Cultural and Ethical Considerations

### 9.1 The politics and the practice

Narrative therapy is the most explicitly political of the approaches in this book, and the practitioner should be able to state both the value of that and the danger of it.

**The value.** The practice's attention to how the dominant discourse constitutes identities is a genuine contribution, and it is what allows a young person who has been labelled to be understood as a person rather than as a label. The observation that a young person's difficulties are frequently produced by the responses of others to them — the school's avoidance, the family's worry, the referral — is clinically important, and the practice gives the practitioner a way of working with it.

**The danger.** The practice's language of the "dominant story" can be turned into an account in which the young person's difficulties are the product of other people's narratives, and in which the young person is thus not responsible for anything. A young person who has been sexually assaulted, or who has developed a serious mental illness, is not a person whose account has been imposed. The practice's own literature has been critical of this tendency in recent years, and the correction is that the story is not the reality, and the events are not the story.

## 9.2 Culture, identity, and the dominant story

**The dominant story is often a cultural story.** A young person who is a member of an ethnic minority in a majority population, who is an immigrant, who is a young person in a family with a particular religious practice, may be living inside a dominant cultural story about people like them, and the practice's externalising questions may be the most direct way of working with that.

**The practice's origins are in a particular intellectual tradition**, and a practitioner who uses the externalising questions should know something about the structuralist and post-structuralist theory and about the feminist critique of the professional claim to know, because the questions make claims and the claims should be able to be defended.

**Naming the problem in a particular way can be culturally loaded.** A name given to a problem in one tradition may carry a specific theological or moral meaning in another, and the practitioner should ask.

**The re-authoring can overwrite the family's or the community's account.** A young person who is invited to author a new account of their life may be implicitly invited to author a *Western individualist* account, and the young person who has a strongly communal sense of self may find the practice strange or offensive.

## 9.3 Ethical requirements

**The consent to externalise is not automatic.** A young person must consent to the practice, and the consent should be informed, and the young person should be told that the practitioner will be asking about the problem in a particular way.

**The confidentiality of the documents.** A letter from the problem contains material the young person may not want a parent or a teacher to read, and the practitioner must establish what may be shared before the document is written rather than after.

**The power of the practitioner over the young person's account.** The practitioner is the one with the questions, and the young person may come to believe that the practitioner has a view about the right account. The discipline is to be genuinely curious, and to notice the times when the practitioner's preferred direction is being imposed.

**Not every problem is a story.** A young person with a medical condition, with a neurodevelopmental condition, or with a substance dependence requires treatment, and the re-authoring is not a substitute for it.

**The practitioner's own position.** A narrative practitioner is not a neutral observer of a story, and the practitioner who is part of a dominant discourse is likely to reproduce it inadvertently, particularly in the questions they think to ask. This should be examined in supervision.

**The social conditions.** The practice's own framework insists that the problem is produced in a social field, and the practitioner who externalises a young person's difficulties while ignoring the conditions in which the young person lives has done the practice's work in name and its opposite in substance. A young person whose problem is a school that is unsafe, or a family that is exhausted by two shifts of work, or a housing situation that is impossible, needs those things changed, and a re-authored story is not a substitute.

# 10. Key Terms

## Key Terms at a Glance

- **Narrative therapy** — The practice developed by White and Epston in which the young person's identity is understood as constituted by the stories they live by, and in which the work is the re-authoring of those stories.
- **The person as the author of their life** — The foundational claim that the account of a life is a draft, revisable, and made by the person whose life it is.
- **Externalisation** — The practice of separating the person from the problem so that the problem becomes an object that can be seen and described, and the person becomes the subject of their own life.
- **The grammar of externalisation** — The shift from "the problem makes you do" to "you do", in which the problem is given the attributes of an agent and the person the attributes of a subject.
- **Naming the problem** — The practice of giving the problem a name, usually the young person's own, and of writing that name down.
- **The problem's influence** — The network of effects the problem has on the young person's life: what it prevents, what it makes them do, and what it costs them.
- **The young person's influence** — The network of effects the young person has on their own life and on others, and the map that is usually thinner than the first and always more useful.
- **Unique outcome** — An event, moment, feeling, thought, or piece of learning that does not fit the dominant story and that opens a space for a different one.
- **The five criteria of a unique outcome** — The young person's own, a specific event rather than a generalisation, a departure from the dominant story, sufficiently detailed to be re-told, and linked to a capacity the young person already has.
- **Dominant story** — The account of the self that the young person lives by, with its subject position, its plot, and its moral.
- **Problem-saturated story** — The older term for the dominant story, in which the young person is the cause of the problem.
- **Re-authoring** — The process of expanding the account of the young person's life so that it includes the unique outcomes, without requiring the negative account to have been false.
- **The difference between a reframe and a re-authoring** — A reframe replaces a negative account with a positive one; a re-authoring expands the account and makes room for the exceptions.
- **The letter from the problem** — A letter written in the problem's voice describing what it has done and what it wants, usually written by the young person.

## Key Terms at a Glance (continued)

- **The letter to the problem** — A letter written by the young person to the problem, in which the young person sets the limits on what it may do.
- **The letter to the young person** — A letter from the practitioner recording what they have noticed and what they value.
- **The take-away letter** — A letter the young person writes to themselves, containing the unique outcomes and the re-authored account.
- **The therapeutic group** — The group in which the re-authoring is done collectively, with the listening restricted to the questions "what was different?" and "how come?".
- **The two permitted questions** — "What was different?" and "How come?", and the exclusion of advice, interpretation, and suggestions.
- **The re-authoring of the unique outcome** — The technique of locating the components of a negative unique outcome and finding them in other areas of the young person's life.
- **The systemic re-authoring question** — "Who else would have to change their story to make room for this?", which connects the individual work to the family and the school.
- **The fabula and the syuzhet** — The raw material of lived events and the organising shape given to them, from narrative theory.
- **The narrative frame** — The structure that selects certain events and excludes others, and the reason that the events excluded from a young person's story are the clinical target.
- **The excised text** — The parts of the story that the frame excludes, and which the practice seeks to recover.
- **Post-structuralist narrative analysis** — The intellectual tradition that treats meaning as made in discourse rather than found in events.
- **Feminist epistemology** — The critique of the professional claim to know the truth about a client, particularly a woman.
- **The problem is not the person** — The reframe that narrative practice offers, and its two corollaries in the externalising work.
- **Meaning is made, not found** — The proposition that the meaning of an experience lies in the account given of it and that the account can be different.
- **The document in the school setting** — The letter from the problem used to convert the teacher's response from a reaction to the young person into a response to the problem.
- **The dominant cultural story** — The account available in a society about a category of person, which the practice's externalising questions can address directly.

**Key Terms at a Glance (continued)**

## 11. Chapter Summary

Narrative therapy emerged in the late 1980s from a combination of literary theory, feminist epistemology, post-colonial thought, and social work practice research, and its central claim is that a person's identity is constituted by the stories they live by and tell about themselves, and that the stories are not the person. The claim is practical rather than abstract: the story through which a young person has organised a genuinely difficult life is now producing the outcomes.

The intellectual sources were described and the development of the movement in Australia and New Zealand was traced, including the sharp break with the family therapy tradition and the prior question the practitioners asked: by what criteria is this family understood to be a family at all.

The core concepts were developed in full. The person as the author of their life was described with its three corollaries — that identity is not fixed, that the problem is not the person, and that meaning is made rather than found. The externalisation was described with its technical vocabulary: the naming of the problem, the writing of the problem's influence, the writing of the problem's influence on the young person's influence on others, the identification of the unique outcomes, and the authoring of a new story. The problem's influence and the young person's influence were described as the two maps, with the observation that the second is almost always thinner and that the discovery of the young person's existing influence is the beginning of a different account.

The unique outcome was described with its five criteria, and the dominant story with its subject position, plot, and moral. The re-authoring was described and distinguished from the positive reframe: a reframe replaces, a re-authoring expands, and the expansion does not require the negative account to have been false.

The techniques were described in detail: the externalising questions and the rules for them; the four documents and their specific uses, in the therapy room, in the school, and in the letter to the young person; and the therapeutic group with its two permitted questions, which are the mechanism of the practice.

The developmental fit with young people was developed through the identity narrative, the externally assigned dominant story, the adolescent's capacity for narrative, and the group's suitability. The initial conversation was described, and the specific modifications for the school and the family were set out, including the systemic re-authoring question. The four obstacles to developing a unique outcome were described with their remedies, including the re-authoring of a negative unique outcome, which is one of the most useful and least discussed techniques in the model.

The case of Keziah demonstrated the model's distinctive operations. The problem was named by the young person and the naming was the externalisation; the unique outcome was located with precision and its components identified; the dominant story was named and the young person's own correction — that other people decide what that means — was the pivot; the re-authoring was done in her own written words; and the letter from the problem gave the teacher and the mother the material to examine the stories they had been telling about her.

The evidence was presented with its unusual structure: the outcome research is among the strongest in the family therapy field, the research on the distinctive outcomes is the most interesting contribution and has identified a consistent set of markers, and the criticism of the methodological quality is legitimate. The strengths and limitations were set out, and the principal limitation — the individualisation of a structural problem — was identified.

The cultural and ethical analysis was substantial. The politics of the practice was examined from both sides: its value in allowing a young person who has been labelled to be understood as a person, and the danger of an account in which the young person is not responsible for anything. The cultural questions were addressed: the dominant story is often a cultural story, the practice's intellectual origins carry commitments that should be known, the naming of a problem can be culturally loaded, and the re-authoring can overwrite a communal account of the self. The ethical requirements closed the chapter: the consent to externalise, the confidentiality of the documents, the power of the practitioner over the young person's account, the recognition that not every problem is a story, the practitioner's own position, and the social conditions, which the practice's own framework insists upon and which a re-authored story cannot substitute for.

## 12. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the three intellectual sources of the narrative therapy movement.
2. State the central claim of narrative therapy and explain why it is practical rather than abstract.
3. State the three corollaries of the person as the author of their life.
4. Describe the practice of externalisation, with its technical vocabulary.
5. Explain the significance of the problem's influence and the young person's influence maps.
6. State the five criteria of a unique outcome.
7. Define the dominant story and state its three components.
8. Explain the difference between a reframe and a re-authoring.
9. State the rules of the externalising questions.
10. Describe the four documents and their uses.
11. Explain the therapeutic group and state the two permitted questions.
12. State the four reasons the approach is used with young people.
13. Describe the initial conversation with a young person.
14. Explain the use of the letter from the problem in a school setting.
15. State the systemic re-authoring question.
16. Describe the four obstacles to developing a unique outcome and the remedy for each.
17. Explain the re-authoring of a negative unique outcome.
18. Describe the case of Keziah and state the four features of it that generalise.
19. Summarise the status of the evidence and the distinctive outcomes research.
20. State the six strengths and the six limitations of the approach.
21. Explain the political dimension of the practice from both sides.
22. State the four cultural considerations for narrative work.
23. State the seven ethical requirements for narrative practice.

## 13. Practice Activities

**Practice and Application**

### Activity 36.1 — The Externalising Question Set (60 minutes, individual)

Write a full set of externalising questions for a specific problem you have encountered, then rewrite each one so that it could only be answered by the problem and not by the person. Then write the variant of each question that would inadvertently re-internalise the problem, and mark it. The exercise teaches the grammar of the practice, and the second half of it teaches the discrimination that distinguishes a practitioner who has internalised the discipline from one who has only learned the questions.

### Activity 36.2 — Finding a Unique Outcome (90 minutes, field-based)

Take a young person from placement and find one unique outcome: a specific event, in the last month, that does not fit the dominant story. Write it out in the young person's words, in the detail in which they described it. Then identify the components — who was present, what was happening, what the young person was doing, what they were wearing, where they were, what the mood of the setting was. Then write the small step that transfers one component into a context where the problem usually operates. The exercise is the core technique of the chapter, and the component analysis is the part that practitioners most often skip and that makes the difference between an outcome and a wish.

### Activity 36.3 — The Therapeutic Group (90 minutes, small group)

In a group of four or five, one member tells a short story of a difficulty, and the others are restricted to the two permitted questions — "what was different?" and "how come?" — for five minutes. Then the storyteller reports what they noticed. The group then processes what it was like to be restricted in this way, and what it was like to receive only those questions. Most groups find the restriction surprisingly hard and the questions surprisingly powerful, and both findings are the point.

### Activity 36.4 — The Dominant Story Audit (75 minutes, individual)

Take a young person from placement and write their dominant story in three sentences, in the third person, as it would be told by the school, by the family, and by the young person. Then write the "who else would have to change their story to make room for this?" answer for each. Then write the practical step for the two people whose stories are most clearly shaping the young person's outcomes. The audit is the exercise that connects the narrative work to the systemic and consultative work of Chapters 33 and 56, and it is the most useful single exercise in the chapter.

## 14. Annotated Reading Pathways

**For the theory.** Read White and Epston (1990) and then White (2007), and read Bruner (1990) for the account of narrative as a mode of human reality. Ricoeur (1984) is the philosophical source and is worth reading in the first part only.

**For the practice.** Read White and Stone (2008) and Epston and White (1995). Then read the group format literature, which is the most immediately usable part of the practice.

**For the intellectual sources.** Read the feminist epistemology literature on the professional claim to know, and read the post-colonial and social constructionist literature on discourse and identity. A student who knows these sources will use the externalising questions with more care and with far less mystification.

**For the evidence.** Read the outcome studies and the distinctive outcomes research, and read the critical appraisals alongside them.

**For the critique.** Read the critiques of narrative practice for its treatment of structural problems and for its individualistic framing, and read the feminist and post-colonial critiques from within the field. The field's own self-criticism is the most useful preparation for working with it responsibly.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Narrative Therapy and the Re-Authoring of Lives** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is externalisation, unique outcomes, and the collaborative reconstruction of adolescent identity. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is family patterns, attachment, boundaries, communication, stories, and relational resources. The mechanism to keep in view is how interaction sequences and shared meanings distribute distress, protection, responsibility, and possibility across a system. The characteristic professional risk is blaming families, exposing a young person in unsafe joint sessions, or ignoring material conditions and individual needs. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in family meetings, school consultations, care networks, and individual sessions informed by systems thinking, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Kofi is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Kofi's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Kofi explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on family patterns, attachment, boundaries, communication, stories, and relational resources. The first intervention is deliberately modest. Kofi chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Kofi has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Kofi can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Narrative Therapy and the Re-Authoring of Lives

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Narrative Therapy and the Re-Authoring of Lives** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Narrative Therapy and the Re-Authoring of Lives** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Narrative Therapy and the Re-Authoring of Lives**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a

discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Narrative Therapy and the Re-Authoring of Lives to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART VI

# Emotional, Trauma-Informed, and Somatic Practice

Theories and methods that work with feeling, with the body, and with the developmental consequences of threat: emotion-focused work, trauma-informed phases, bilateral stimulation, and somatic regulation.

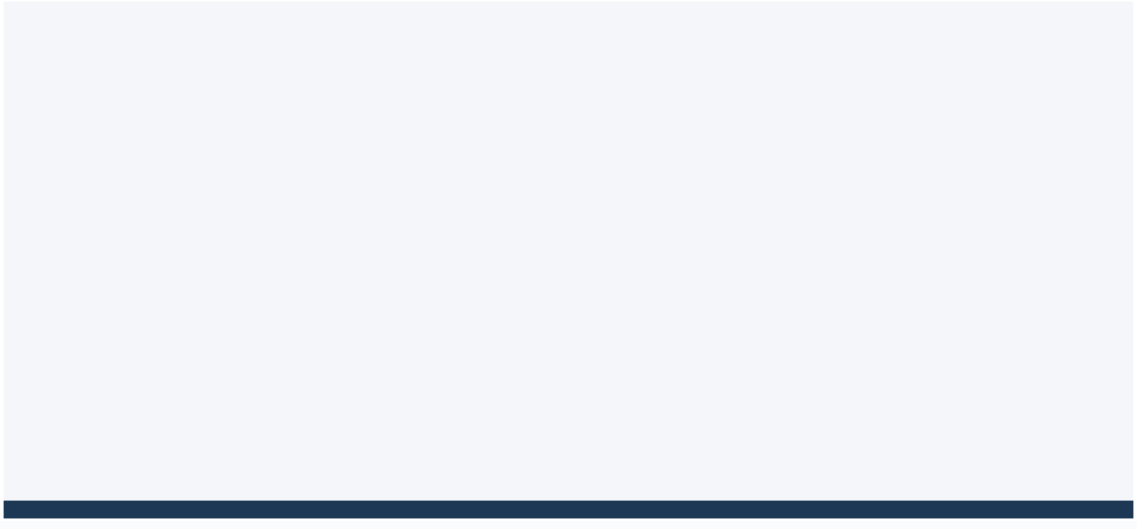
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CHAPTER 37

# Emotion-Focused Therapy

Greenberg's model of emotion regulation, the change point, and emotion-focused work with youth in distress.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of emotion-focused therapy and describe the clinical problems that produced it.
- State the principles of the emotion-focused model, including the position that emotion is the primary datum in therapy.
- Describe the model of emotion as a response to a need, and the categories of emotion the model identifies.
- Explain the concept of the change point and the two components of the process.
- Describe the emotional processing steps and the in-session emotion coaching.
- Apply the emotion-focused framework to a case involving an adolescent in emotional distress.
- Critically evaluate the cultural and ethical dimensions of emotion-focused work, including the questions of the cultural status of emotional expression and of the relationship between emotion and need.

## Chapter Overview

Emotion-focused therapy, in the form Leslie Greenberg and colleagues developed it, is unusual in this book in making the client's **emotional experience** rather than the client's thoughts, behaviour, or relationships the primary object of the work. Its founding observation was clinical and unremarkable: **clients change when they are able to identify, experience, and express their core emotions, and they do not change when they do the other things first.**

That observation was in part a reaction to a specific and widely felt failure of the cognitive therapies. Some clients arrive in cognitive therapy able to dispute their thoughts perfectly and leave unchanged, and the reason is that the problem is not in the thoughts. The client believes "I am worthless" and disputes it accurately — "I have got a job, I passed my exam, my aunt says I'm fine" — and the dispute changes nothing, because the feeling of worthlessness is not generated by the belief and is not removed by its refutation. The feeling is generated by an unmet need, and the needs are the clinical target.

The model has two further features that make it valuable with young people. The first is that it is **brief and focused**: the treatment has a limited number of sessions, a specific target emotion, and a specific procedure, and a young person who has been offered years of weekly sessions that produce no change is often well served by an approach that names a target and a timeframe. The second is that it is **developmentally consonant with the adolescent's task**: the adolescent's central developmental work involves the construction of a new self, and the emotions of that period — the shame, the anger, the grief, the anxiety — are the raw material of the work of becoming an adult, and the model treats them as such rather than as symptoms to be eliminated.

The chapter presents the model, the theory of emotion, the change point, the processing steps, the developmental adaptations, and the critical analysis, which includes the question of whether the model's account of emotion is culturally universal, and it is not.

## 1. Historical Context

### 1.1 The two foundations

The model has two intellectual sources, and both are in this book already.

**The client-centred tradition of Chapter 10.** The empathic attunement, the congruence, and the understanding of the client's frame of reference, and the finding that the conditions of the relationship are themselves therapeutic.

**The cognitive tradition of Chapter 16.** The technique of disputing maladaptive thoughts, which Greenberg did not abandon but repurposed: the cognitive disputation becomes one of several ways of responding to an emotion, and it is the *least* effective of them for most clients.

### 1.2 The development

Greenberg and colleagues began the work in the late 1970s at the University of British Columbia, initially as a research programme comparing a structured, emotion-focused treatment with a structured cognitive treatment for depression. The design was unusual in asking the researchers to code the process of therapy in detail, and the coding produced the finding that changed the field: **emotional change, rather than cognitive change, was the best predictor of improvement**, and the improvement in emotion in the session was the best single predictor of improvement in the session.

The programme then developed into a series of controlled studies that established the specific procedures, and the model was extended from depression to a wide range of presentations, including the anxiety disorders, the eating disorders, the interpersonal problems, the trauma-related presentations, and the problems of couples, and it was adapted for young people.

### 1.3 The place in the field

Emotion-focused therapy is not a mainstream school, and its position is unusual: it has a good research base, a well-specified protocol, and relatively limited clinical adoption. A student should be aware of the reasons, and they are instructive.

**The model requires a level of emotional processing that not every client can reach.** The model's procedures depend on a client who can access, identify, and stay with a core emotion. A young person with a very restricted emotional range, with a very high level of dissociation, or with a very limited capacity for reflection may not be able to do what the model asks, and the field's own position is that these presentations need a longer course of preparatory work.

**The model requires a trained practitioner.** The procedures look simple and are not, and a badly delivered emotion-focused session in which the practitioner pushes the client toward a feeling the client cannot bear is not a neutral event.

**The model's account of emotion is under-specified.** The theory of emotion as a response to a need is elegant and is not a theory of emotion in the sense that the affective science would recognise, and a student who wants a developmental account of the emotional system should read the affective science as well as Greenberg.

## 2. The Core Principles

### 2.1 Emotion as the primary datum

The model's foundational claim is that **emotion is the primary datum in therapy**, and that the other elements — cognition, behaviour, and the relationship — are in the service of emotion.

The claim is not that cognition is unimportant. It is that the cognitive intervention is subordinate: a belief is examined because it is generating or blocking an emotion, and a behaviour is examined because it is serving an emotional function, and the alliance is examined because a rupture in it will block the emotional work.

## 2.2 The nature of emotion in the model

Greenberg's account of emotion has four components, and a student should be able to state them.

**Emotion as a response.** Emotion is a response to a need — the need for a secure attachment, the need for autonomy, the need for control, the need for approval, the need for a sense of worth, the need for a connection with others, the need for a sense of identity. The primary emotions are organised in relation to these needs, and the specific emotion reflects the specific need and the specific way in which it is being frustrated.

**Emotion as adaptive.** Every emotion serves a purpose. Anger mobilises against a violation; shame mobilises against exposure; sadness mobilises against loss; fear mobilises against threat. The problematic emotions are not the ones that are wrong in themselves but the ones that are misdirected, excessive, or aimed at the wrong target.

**Emotion as tolerable and changeable.** Healthy emotions are felt, expressed, and then resolved. A primary emotion that is adequately processed ends; the problem arises when a secondary emotion — the feeling that arises in the reaction to the primary one — blocks it. Guilt is a secondary emotion that typically arises in the reaction to anger, and it blocks the anger and the anger's work.

**Emotion as transcending.** The most developed part of the model's account is the proposal that the highest form of emotional life is a form of transcendence in which the self is expanded beyond the narrowness of the immediate situation, and the "solution" emotions in the model — pride, confidence, calm, and a sense of the other as a potential ally — are understood as the forms of emotional transcendence.

## 2.3 The categories of emotion

The model's categories of emotion are clinically useful and a student should be able to use them.

**Primary emotions** are the adaptive responses to the frustration of a need: anger, sadness, fear, shame, joy, and the physical sensations that accompany them. They are "maladaptive" only when they are misdirected, excessive, or persistent.

**Secondary emotions** are the feelings that arise in the reaction to the primary ones, and they function to block the primary emotion's processing: guilt typically blocks anger, depression blocks anger and grief, shame blocks pride, and anxiety blocks the approach that the underlying need is demanding. The clinical target is very often the secondary emotion, because it is the secondary emotion that is generating the presenting complaint.

**Emotion schemes** are a generalisation from a specific emotional response, and they are the level at which the model locates the enduring vulnerability: "if I am not in control, I will be helpless"; "if I am rejected, I am worthless"; "if I fail, I am a failure".

**Emotional responses and their variations** — the way in which a particular emotion is expressed and experienced, which is a matter of the person's habitual response rather than of the emotion itself.

### 2.4 The position on the relation between emotion and cognition

The model is clear that **cognition and emotion are mutually determining** and that the direction of determination varies from person to person and from moment to moment. Some clients change when they think differently; others change when they feel differently; and many need both.

The clinical implication is diagnostic rather than theoretical: the practitioner must determine, for this client and at this point, which is the bottleneck. Greenberg's own phrasing is that the practitioner should ask whether the client's problem is primarily a thinking problem, a feeling problem, or a doing problem, and the answer determines the intervention.

## 3. The Change Point

### 3.1 The concept

The **change point** is the specific point in the processing of a core emotion at which the client is most likely to change, and it is the most distinctive technical contribution of the model. The concept is derived from the observation that clients are most amenable to change at a particular moment in the emotional process, and that practitioners who are not aware of the moment will either press too hard too early or press too softly too late.

**The change point is the moment when the client is experiencing the core emotion at its most, and is simultaneously able to remain present with it.** Before the change point, the client is either not in the emotion or is being protected from it by a secondary emotion. After the change point, the emotion has been resolved or has been abandoned in favour of a secondary emotion or a defence.

The change point is thus a narrow window, and the practitioner's task is to find it and to work in it.

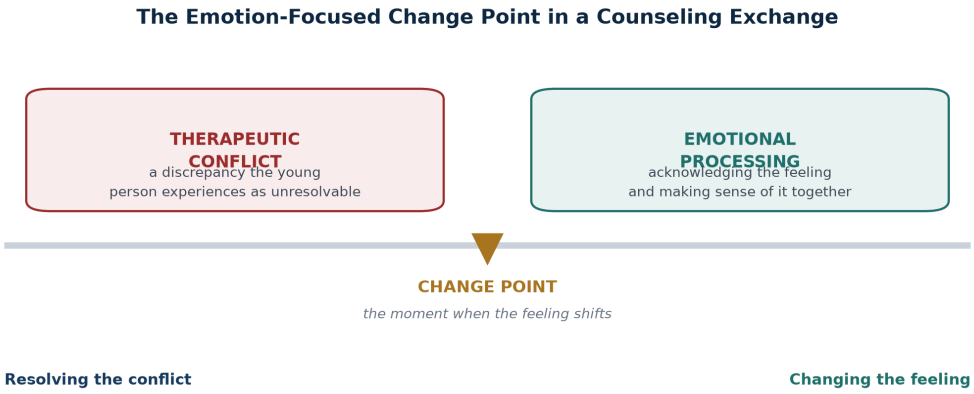



**Figure 37.26** The change point located within the arc of the emotional process, showing the primary emotion, the secondary emotion that blocks it, the point of maximum emotional arousal, and the resolution.

### 3.2 The two components

The change point has two components, and a practitioner who recognises only one of them will miss the moment.

**The emotional component** is the client's experiencing of the core emotion at its peak, with the associated physiological arousal, the facial expression, the voice quality, and the body posture.

**The cognitive component** is the client's accessing of the core appraisal — the meaning of the situation for the core need. The appraisal is what determines the emotion, and a client who is feeling the emotion at its peak but who has not accessed the appraisal is in a state of arousal without a meaning, and the state will be resolved by a secondary emotion or a defence rather than by a reorganisation.

The two components occur together in the moment of the change point, and a practitioner who produces the emotional component without the cognitive component has produced a discharge rather than a change.

### 3.3 The practitioner's task

The task is a sequence, and it is worth stating it as a set of steps.

**Identify the core emotion.** The work begins in the assessment, and the identification of the core emotion is one of the deliverables of the assessment phase.

**Find the blockage.** Almost always a secondary emotion, and the task is to identify it and to name it for the client.

**Work through the blockage.** The practitioner does this with warmth and with some assertiveness, and the standard move is to say what the practitioner thinks the client is feeling and to invite the client to consider it. This is where the "empty chair" and the "chair work" of the process stage becomes relevant.

**Achieve the change point.** The client is then helped to stay with the core emotion, and the practitioner supports the arousal, and the practitioner's own arousal and warmth are the instruments.

**Facilitate the resolution.** The resolution is not the end of the session; it is the beginning of the new emotional state, and the practitioner helps the client experience the solution emotion.

### 3.4 The emotional processing steps

The standard procedure for the processing stage has five steps, and a student should be able to name them.

1. **Attune and focus.** The practitioner attends to the core emotion, usually by reflecting it, and sometimes by exaggerating it, and the exaggeration helps to bring the emotion into focus.
2. **Confirm and clarify.** The practitioner confirms the emotion, sometimes by naming it directly, and the naming is a key moment, because a client who has never had their core emotion named by another person is being given a new experience.
3. **Congruence and the empathic stance.** The practitioner is present with the emotion rather than managing it, and this is the "being with" stance, and it is a demanding stance to hold.
4. **Facilitating the expression.** The practitioner encourages the expression of the core emotion, and the expression is the vehicle of the processing, and the practitioner matches the arousal.
5. **Reaching the resolution.** The emotion resolves, and the practitioner helps the client notice the new emotional state, and the noticing is essential, because the new state is the thing that will be available to them in the future.

## 4. The In-Session Emotion Coaching

The second major procedure, and the one most applicable to work with young people, is the **emotion coaching**, and it comes from a different research tradition — the work on parental emotion regulation with young children, and the body of research on the "emotion socialization" of children.

### 4.1 The procedures

**The empathic statement.** The practitioner reflects the child's or young person's emotional state in a statement, and the statement is a hypothesis that the young person can correct, and the correction is itself data.

**The emotion labelling.** The practitioner names the emotion, using a label from a list that is introduced gradually and is not pejorative. "Frustrated" is a safer first label than "furious", and "disappointed" is safer than "sad".

**The validation.** The practitioner validates the emotion by stating its legitimacy in the situation. The validation is of the emotion, not of the behaviour, and the distinction matters, and a practitioner who validates the behaviour has made an error.

**The problem-solving and the choices.** The practitioner helps the young person identify the problem, and then the choices. The order is important: the emotion is processed first and the problem-solving second, and reversing the order produces a young person who solves the problem without ever having been with the feeling that generated it.

### 4.2 The emotion coaching with parents

The most widely used application of the emotion coaching is with the parents of young children, and it is effective enough that a student should know it even if the population served is older. The structure is a hierarchy of responses to a child's distress: first, the parent is taught to **empathise** and to name the emotion; second, to **validate** the emotion; third, to **label** the emotion and set a limit on the behaviour if necessary; and fourth, to **problem-solve**.

The teaching is done with video-feedback where possible, and it is the first and the third steps that parents most often skip, and they are the steps that matter most.

### 4.3 The emotion coaching with adolescents

With an adolescent, the same procedure is used with two modifications, and the modifications are what make it usable.

**The developmental modification.** The adolescent is not told what the emotion is and is not told what to do; the practitioner labels and validates and then asks. The adolescent's own label is preferred, and the adolescent's own solution is required, because a solution supplied by an adult is a solution the adolescent will not use.

**The relational modification.** The emotion coaching is often best delivered through the parent, because the adolescent's capacity to be with their own emotions is being undermined by the parent's response to them. A young person whose mother's response to her distress is to become more distressed cannot learn to self-soothe, and the work with the mother is the prerequisite.

## 5. Further Reading for This Chapter

- Greenberg, L. S., Rice, L. J., & Greenberg, J. S. (Eds.). (1997). *Facilitating emotional change: The moment of change in brief therapy*. — The foundational clinical text.
- Greenberg, L. S., & Rice, L. J. (2001). *Facilitating emotional change: The moment of change in brief therapy*. — The process study.
- Greenberg, L. S., Rice, L. J., & Rivera, R. (2003). *Facilitating emotional change: The moment of change in brief therapy*. — The emotion-focused model.
- Greenberg, L. S., & Paiv, S. T. (1997). *Motivational interviewing: Expressive and instrumental functions*. — The argument for the discrepancy between motivational and emotional work.
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- Greenberg, L. S., & Webster, S. (1995). *Resolving emotional conflicts: The client as the active ingredient*. — The change point.
- Greenberg, L. S., Ford, M. D., & Lawrence, J. (2004). *Facilitating emotional change in the context of a dialogue*. — The structure of the process.
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- Greenberg, L. S., & Elliott, W. (2002). *The therapeutic relationship and the process of change*. — The research on the alliance and the change point.
- Cooper, M., & Golan, S. (2013). *The emotional life of your teenager*. — The emotion coaching with adolescents.
- Gottman, J., & Declaire, J. (1997). *Raising an emotionally intelligent child*. — The parental emotion coaching.
- Gershen, R. (2012). *The effect of emotion regulation on the therapeutic process*. — The research.
- Hadley, S. W. (2014). *Theories of counseling and psychotherapy*. — The wider clinical context.
- Safran, J. D., & Muran, J. C. (Eds.). (2006). *The therapeutic relationship*. — The research context.
- Greenberg, L. S. (2015). *The research and practice of emotion-focused therapy*. — The contemporary statement.
- Koenig, J., & Dyer, J. (2020). *The therapeutic relationship in emotion-focused therapy*. — The mechanism research.


# 6. Emotion-Focused Work with Children and Adolescents

## 6.1 The developmental fit

Emotion-focused therapy is unusually well matched to the adolescent period, and a student should be able to give the reasons.

**Adolescence is the period of the highest emotional intensity in the human life course.** The affective neuroscience of the period is fairly consistent: the reactivity of the emotional system is at its peak, the regulation capacity is still developing, and the gap between the two is at its widest. A model that works directly with emotion is working with the adolescent's actual experience rather than around it.

**The developmental task of adolescence is the construction of a new self, and the emotions are the material.** The shame, the grief for the childhood, the anger at the limits, the anxiety about the future, the exhilaration of the first relationships — these are not symptoms. They are the content of the developmental work, and the model treats them as such.

**The adolescent's capacity to reflect on their own emotions is newly available.** The concrete operational stage of Piaget's sequence and the corresponding increase in metacognitive capacity mean that a young person at fourteen or fifteen can, in the right conditions, do the work the model asks. This is a substantially different picture from the child of eight, and it is why the model's adolescent adaptations are viable.

**Emotion-focused work is a good fit for the presentations that most often reach a school or community service.** The young person who is angry, who is ashamed, who is grieving, who is anxious, who cannot say what they feel, and who is in trouble — these are the presentations in which the model's core question is most directly useful: what is the core emotion, and what need has it arisen from?

## 6.2 The specific adaptations

**The assessment phase is longer and the targets are negotiated.** The model assumes an assessment in which the core emotion and the pattern are identified. With an adolescent, the practitioner should expect to spend two or three sessions on the assessment, and should be honest with the young person about the length, and should be explicit that the assessment is not a delay before the work but a part of it.

**The core emotion is often not the presenting emotion.** A young person who presents with anger may have a core emotion of shame, and a young person who presents with anxiety may have a core emotion of grief. The assessment's task is to get beneath the presenting emotion, and the presenting emotion is frequently the secondary emotion that is doing the blocking.

**The work with the parents is essential and is often the difference between success and failure.** This is the single most important developmental modification. The adolescent's capacity to be with their own emotions is being continuously undermined by the responses of the adults around them, and the work with the parents is a prerequisite for the individual work rather than an adjunct to it. A practitioner who works with a distressed adolescent in a household where the adults cannot tolerate the adolescent's distress is doing work that will not hold.

**The pace must be slower and the arousal lower.** The change point requires the client to sustain a high level of arousal, and a young person with a dysregulated nervous system — and many do — cannot sustain it, and pushing for the change point with a young person who is already at their limit produces a collapse rather than a change. The preparation phase of Chapter 40 is not optional for many of the young people who present.

**The resolution must be named and recorded.** The new emotional state at the end of the session should be explicitly noticed, and where possible written down, and a young person who has had a peak of distress followed by a moment of calm has had an experience that is worth keeping.

## 6.3 The presentations

Four presentations are particularly suited to the model.

**The young person with a chronic anger problem.** The anger is nearly always a secondary emotion blocking a primary emotion, usually grief or shame, and the treatment is to work beneath the anger rather

than to manage it.

**The young person with a physical presentation.** A young person who comes to the clinic with headaches, with abdominal pain, with a rash, with a sleep disturbance, and with no medical explanation, may be presenting a primary emotion in a somatic form, and the somatic work of Chapter 43 is the natural companion to this chapter.

**The young person with an eating presentation.** The emotion-focused model has been applied to both the restrictive and the bingeing presentations, and the eating behaviour is typically functioning to manage a primary emotion that the young person cannot otherwise tolerate.

**The young person with a relational problem.** The model's interpersonal procedure, in which a specific interaction with a significant other is imagined and worked through, is particularly useful with the young person whose difficulties are concentrated in a relationship with a parent, with a peer, or with a partner.

### 6.4 The limits

**The model requires the capacity to be with a feeling.** A young person with a very restricted range, with high dissociation, or with a severe and longstanding trauma response may not be able to access and sustain the core emotion, and the preparatory work of Chapter 40 must come first.

**The model can be experienced as a demand to feel.** A young person who has been told by adults to "calm down" for fifteen years may experience the invitation to sit with a feeling as another demand, and the practitioner should name this possibility in advance.

**The model can be experienced as the practitioner doing the emotional work.** The stance of "being with" the client's emotion is a demanding stance for the practitioner, and the practitioner who is managing their own distress rather than sharing it will be detected, and a young person will stop bringing the feeling.

## 7. Case Illustration: The Young Woman Who Was Always Angry

**Presentation.** Tia is fifteen, and the referral from school describes a young woman who is "aggressive with staff, swearing, and defensive". Her mother describes a young woman who "explodes over nothing" and who has become "a different person since her grandmother died in July". The school has had three exclusions.

**The first session.** The practitioner begins with the emotion coaching, not with the assessment, and the reason is that the assessment will not proceed with a young person who is in the state Tia is usually in.

The practitioner says: "I want to start with something simple. You've come in looking like you're ready to fight, and I wonder if that's actually about something else." Tia says, sharply: "It's about whatever you want to make it about." The practitioner reflects: "So it's about not being messed with." Tia says: "It's about being *treated*." And then, unexpectedly, she tells the practitioner that the school has been telling everyone she is a "problem" and that she has read the comments in the file, and that her own mother has said "you were exactly like your dad" and that her dad is in prison.

**The second session.** The practitioner returns to the anger. Tia says: "Everyone expects me to be angry, so I am." The practitioner asks what she is underneath it, and Tia says: "Tired." And the practitioner works with the tired for a while, and Tia says: "I am so tired of being the one that's wrong, and I can't even be sad about

Gran because then I'm the one who's sad and everyone's got enough."

**The assessment.** Over the next two sessions, the practitioner works on the assessment, and the assessment identifies the pattern with a precision that surprises Tia.

The core emotion is **grief**, for a grandmother who died in July after a long illness, and Tia had been the primary carer, having spent most of her fifteenth year in hospitals and at her grandmother's bedside.

The secondary emotion is **anger**, which is what Tia has been presenting, and the anger has been blocking the grief, and the anger has been doing two things: it has kept Tia from having to feel the grief, and it has been the only available response to a set of situations in which Tia is treated as the problem — including, most significantly, by her own mother, whose grief for her own mother has been expressed as anger at Tia.

The appraisal at the change point is the meaning of the loss: that the people she cared for are taken, and that nobody has asked her about it, and that being angry is the only way she has found to be listened to.

**The intervention.** Four elements.

*Element one: the reframe for Tia.* The practitioner does this explicitly and in a direct way: "The anger isn't the problem. The anger is what you have been doing instead of the grief, and it is the only way you have found to get anybody to listen. And I want to say something else, which is that the things people have said to you — the comments in the file, your mum saying you're like your dad — those are not descriptions of you. Those are them not knowing what to do with a young woman whose gran died."

*Element two: the work with the mother.* This is the part of the plan on which the outcome will turn, and it is the hardest. The practitioner sees the mother alone, and the mother's account is that she has been "strong for Tia" and has not cried in front of her. The practitioner asks the mother about her own mother, and the mother cries, and the practitioner stays with her, and this is the first time anyone has asked the mother about her own loss. The specific change requested of the mother is: to stop the comparison with Tia's father entirely, and to tell Tia once, directly, that she is grieving too, and to stop asking Tia to be "strong" or "the strong one".

*Element three: the emotional work with Tia.* The practitioner works toward the change point over two sessions, with the preparation work of the previous sessions doing the work of establishing that Tia can stay with a feeling. The peak arrives in the fourth session, when Tia says, in a voice the practitioner has not heard from her: "I miss her. I miss her so much and nobody will let me say it because they all think I'm angry." The practitioner stays with her, does not interrupt, and does not reassure, and Tia cries for a long time, and at the end of the session Tia is quiet, and the practitioner asks what she is noticing, and Tia says: "I think I'm just tired, but it's a different tired to before."

*Element four: the school.* The practitioner writes to the school with Tia's agreement, and the letter is short and it says that Tia is grieving, that the exclusions are not helping, and that the school could be part of the treatment by allowing Tia to be absent for two days for the funeral that Tia was not permitted to attend. The school agrees, and Tia attends the funeral, and the practitioner does not overstate the significance of this, because Tia is not yet over anything.

**The outcome.** Over the following two months, Tia has one further exclusion, which follows an incident in which a younger student made a comment about her grandmother. Tia's mother tells the practitioner that they have been to the cemetery together twice. The school reports that Tia is engaging in lessons. The anger has not gone, and the practitioner says clearly that it is not expected to go, and that what has changed is what it is for.

**The interpretation.** The case illustrates the model's central technique. The presenting emotion was not the core emotion; the core emotion was identified through the secondary; the change point required both the emotional and the cognitive components, and the practitioner had to build the conditions for the cognitive component — the appraisal that the loss meant and that nobody would listen to it — before the peak arrived; and the most important intervention was not with Tia at all but with her mother, whose grief and anger were sustaining the pattern. The model predicts that the individual work will not hold without the systemic work, and this is the model's most practically useful claim for work with young people.

# 8. Evidence and Research Support

## 8.1 The status of the evidence

Emotion-focused therapy has one of the better research bases in the psychotherapy literature, and the student should be able to describe its structure.

**The process research is the strongest part of the base.** The finding that emotional change in session predicts improvement, and that it predicts it better than cognitive change, has been replicated across a range of presentations and across a range of therapists, and it is one of the more robust process findings in the field.

**The controlled trials are good.** The randomised trials of emotion-focused therapy against cognitive therapy and against treatment as usual have reported significant effects, with the emotion-focused treatment at least equal to the cognitive treatment and sometimes superior, and the effect sizes are large.

**The brief treatment claim is supported.** The model was developed as a brief treatment, with a specified number of sessions, and the trials support the efficacy of the time-limited protocol.

**The evidence base is smaller than that of the cognitive and behavioural therapies**, and the reason is partly the difficulty of the fidelity measurement and partly the smaller number of practitioners.

**The adolescent evidence is limited.** Most of the research has been with adults, and the developmental adaptations have been developed largely from clinical experience and from the related emotion-coaching research with parents of young children. A student should be candid about this.

**The emotion coaching research base is good.** The parental emotion coaching research has a substantial evidence base for reducing the emotional dysregulation of young children, and the effects are mediated by the parents' own regulation capacity.

## 8.2 Limitations of the evidence

**The trials are mostly by the model's developers and their close colleagues**, and the allegiance effects are correspondingly large.

**The fidelity measurement is difficult**, and the procedures are detailed enough that delivery varies considerably.

**The adolescent and the clinical populations with the most complex presentations are under-researched.**

**The measure of emotion change in session is a judgement-based rating**, and the research on the predictive power of it is robust but the construct is not independent of the observer's expectation.

# 9. Strengths and Limitations of Emotion-Focused Therapy

## 9.1 Strengths

**It is direct and it is brief.** A specified number of sessions, a specified target, and a specified procedure are all things a young person can be told, and a young person who is tired of being talked at will often prefer it.

**It treats the emotion as data.** The model does not pathologise the feeling, and a young person who has been told that their anger is a symptom is often relieved to discover that the anger is a response.

**It is compatible with everything else in this book.** The emotion-focused work can be combined with behavioural, cognitive, systemic, attachment, and trauma-informed work, and the combination is often the most useful.

**It identifies the secondary emotion.** This is a genuinely useful clinical skill, and it applies well beyond this model: the presenting complaint is frequently not the target.

**It locates the need.** The move from "what is the problem?" to "what need is this emotion in service of?" is one of the most useful questions a practitioner can ask, and it is applicable in almost any context.

## 9.2 Limitations

**It requires a capacity that some young people do not have**, and the preparatory work it assumes is not always done.

**The model is under-specified in its account of the emotional system.** It is a clinical model of emotion rather than a developmental or a neurological one, and a student who wants the latter should read the affective science.

**It can be experienced as a demand to feel.** The work with the emotion has to be prepared for and negotiated.

**The research base is narrow.** The adolescent evidence in particular is thin.

**It is vulnerable to practitioner error.** The procedures are simple and the pitfalls are not.

# 10. Cultural and Ethical Considerations

## 10.1 Culture and the expression of emotion

The model's account of emotion is universalist, and a student should be able to state where it is culturally bounded.

**The expression of emotion is culturally regulated.** In many cultures, particular emotions — anger, in particular, and in some contexts sadness and grief — are subject to strict regulation, and the display of them brings consequences. A young person from a family in which the expression of anger is dangerous will not access the anger in the consulting room, and the practitioner who works toward the change point regardless will produce a young person who complies in the room and shuts down outside it.

**The display rules differ and the practitioner must learn them.** The rules about what may be expressed, to whom, and in what form are learned early and are enforced by the family and the community. The practitioner's task is to establish them before the work, and the question to ask the family is not "how does

this young person express anger?" but "what happens in your family when someone is angry?"

**The categories of the model are themselves culturally patterned.** The taxonomy of the primary, secondary, and solution emotions was developed in a specific clinical culture with a specific repertoire of expressed affect, and the mapping of a young person's experience onto it requires a translation, and the translation should be done with the young person rather than to them.

**Grief and its rituals.** The model works with grief, and the rituals of grief are culturally determined and often specific, and a young person whose family's mourning practice is elaborate and public will need the practitioner to work within it rather than to substitute a Western individual process.

## 10.2 The politics of the model

The model is individual. The central question — "what emotion is this, and what need is it in service of?" — locates the difficulty in the young person's own emotional life, and a practitioner who asks it relentlessly can inadvertently individualise a difficulty that is produced by the young person's circumstances.

The correction is straightforward and the practitioner should be able to state it: **the need is real and the response of the environment to the need is also part of the picture.** A young person whose need for safety is frustrated by a school that is unsafe is not to be helped by processing the emotion; she is to be helped by the school becoming safe. A young person whose need for a stable home is frustrated by unstable housing is not to be helped by attending to the grief.

## 10.3 Ethical considerations

**The consent to the work.** A young person should know what the treatment involves, including that the practitioner will be asking them to stay with difficult feelings, and this should be said in advance.

**The regulation of the arousal.** The change point requires a high level of arousal, and a young person with a dysregulated nervous system, with a seizure disorder, with a cardiac condition, or with a history of an overwhelming response needs the preparatory work of Chapter 40 first, and the practitioner must screen for this rather than discover it in the session.

**The limits of the practitioner.** The "being with" stance is a demanding one, and the practitioner needs their own support and their own regulation, and a practitioner who is dysregulated will dysregulate the client. This is a supervision matter and it is a matter of professional self-care.

**The relationship with the parents.** The work with the parents must be transparent to the young person, and the boundaries between what is shared and what is not must be explicit, and the young person must be told what is said about them.

**The prohibition on the emotional pressure of the practitioner.** A practitioner who is Determined to reach the change point in a particular session is applying pressure, and the pressure is a form of coercion, and it can produce a compliance that is not a processing.

**Confidentiality and the disclosure of the core emotion.** A young person may disclose a core emotion in a session that the practitioner shares with the parents, and the disclosure is the treatment, and it must be protected.

# 11. Key Terms

## Key Terms at a Glance

- **Emotion-focused therapy** — The brief, structured treatment developed by Greenberg and colleagues in which the client's emotional experience is the primary datum and the focus of the work.
- **Emotion as the primary datum** — The model's foundational claim that emotion, rather than cognition, behaviour, or the relationship, is the primary object of the work.
- **Emotion as a response to a need** — The account in which the primary emotions are organised in relation to the needs for secure attachment, autonomy, control, approval, worth, connection, and identity.
- **The needs framework** — The seven needs in relation to which the primary emotions are organised, and the clinical question of which need a given frustration has activated.
- **Emotion as adaptive** — The proposition that every emotion serves a purpose, and that the problematic emotions are misdirected, excessive, or aimed at the wrong target.
- **Primary emotions** — The adaptive responses to the frustration of a need: anger, sadness, fear, shame, joy, and the sensations that accompany them.
- **Secondary emotions** — The feelings that arise in the reaction to the primary ones and that function to block the primary emotion's processing: guilt, depression, shame, and anxiety.
- **The blockage** — The secondary emotion that prevents the processing of the core emotion, and the usual target of the work.
- **Emotion schemes** — A generalisation from a specific emotional response, and the level at which the model locates the enduring vulnerability.
- **Solution emotions** — Pride, confidence, calm, and the sense of the other as a potential ally, understood as the forms of emotional transcendence.
- **Emotional transcendence** — The highest form of emotional life, in which the self is expanded beyond the narrowness of the immediate situation.
- **The change point** — The moment in the processing of a core emotion at which the client is most amenable to change, and the model's central technical contribution.
- **The two components of the change point** — The emotional component, the experiencing of the core emotion at its peak, and the cognitive component, the accessing of the core appraisal.
- **The core appraisal** — The meaning of the situation for the core need, which determines the emotion and which must be accessed for a change to occur.
- **The emotional processing steps** — Attune and focus, confirm and clarify, the empathic congruence, facilitate the expression, and reach the resolution.

**Key Terms at a Glance (continued)**

- **The "being with" stance** — The practitioner's congruent presence with the client's emotion rather than its management.
- **Emotion coaching** — The procedure of empathising, labelling, validating, and then problem-solving with the client, and the strategy of doing so with parents of young children.
- **The empathy step** — The practitioner's empathic statement of the client's emotional state, offered as a hypothesis the young person can correct.
- **The labelling step** — Naming the emotion with a non-pejorative label, introduced gradually.
- **The validation step** — Stating the legitimacy of the emotion in the situation, and of the emotion rather than the behaviour.
- **The problem-solving step** — Identifying the problem and then the choices, and the requirement that it come after the emotion is processed.
- **The parental emotion coaching hierarchy** — Empathise, validate, label and limit, and then problem-solve.
- **The thinking, feeling, and doing question** — The diagnostic question of whether the client's problem is primarily a thinking, a feeling, or a doing problem, and the intervention it determines.
- **The focus of attention procedures** — The set of techniques in which the client is guided in attending to a particular aspect of their internal experience.
- **The intrapersonal technique** — The processing of the client's experience of their own emotion in the presence of the practitioner.
- **The interpersonal technique** — The processing of a specific interaction with a significant other, imagined and worked through in the session.
- **The preparatory phase** — The work on regulation and stabilisation that must precede the processing phase for a young person with a dysregulated nervous system.

## 12. Chapter Summary

Emotion-focused therapy is a brief, structured treatment in which the client's emotional experience is the primary datum, and its founding observation — that clients change when they can identify, experience, and express their core emotions — was a direct response to the failure of the cognitive therapies with clients whose difficulties are not in their thoughts. The chapter traced the two foundations, the research programme in British Columbia that identified in-session emotional change as the best single predictor of improvement, and the reasons for the model's relatively limited clinical adoption, which are instructive: the requirement for a capacity to access feeling, the requirement for trained practitioners, and the under-specification of the account of emotion.

The core principles were developed: emotion as the primary datum, with the other elements in its service; the four components of the model's account of emotion, as a response to a need, as adaptive, as tolerable and changeable, and as transcending; the categories of primary, secondary, and solution emotions and the

emotion schemes, with the observation that the clinical target is very often the secondary emotion because it is the secondary emotion that generates the presenting complaint; and the position on the mutual determination of cognition and emotion, with the practical diagnostic question of whether the problem is a thinking, a feeling, or a doing problem.

The change point was described in detail, with the concept, the two components, the practitioner's task as a sequence, and the five steps of the emotional processing. The critical technical point was made that a practitioner who produces the emotional component without the cognitive component has produced a discharge rather than a change.

The in-session emotion coaching was described with its four procedures and the parental hierarchy, and the two developmental modifications that make it usable with adolescents — the adolescent's own label and the adolescent's own solution, and the use of the coaching through the parent where the parent's response is undermining the adolescent's capacity.

The developmental fit was developed through the four reasons, and the specific adaptations were described: the longer and negotiated assessment, the identification of the core emotion beneath the presenting emotion, the essential work with the parents, the slower pace and lower arousal, and the naming and recording of the resolution. The four suited presentations and the three limits were given.

The case of Tia demonstrated the model. The presenting anger was identified as a secondary blocking a grief, the appraisal at the change point was the meaning of the loss and of not being listened to, the practitioner built the conditions for the cognitive component before the peak arrived, and the decisive intervention was with the mother, whose grief and anger were sustaining the pattern. The model's prediction that the individual work will not hold without the systemic work is its most practically useful claim for work with young people.

The evidence was presented with its structure: the process research is the strongest part of the base, the controlled trials are good, the brief treatment claim is supported, the parental emotion coaching research is good, and the adolescent evidence is limited and developed largely from clinical experience. The limitations were named.

The strengths and limitations were set out, with the strongest strengths being the directness, the brevity, the treatment of emotion as data, and the identification of the secondary emotion, and the principal limitations being the capacity the model requires, the under-specified account of emotion, the thin adolescent evidence, and the vulnerability to practitioner error.

The cultural analysis addressed the cultural regulation of emotional expression, the need to learn the family's display rules before the work, the cultural patterning of the model's own categories, and the cultural specificity of the rituals of grief. The politics of the model were addressed with the correction that the need is real and the environment's response to it is also part of the picture. The ethical requirements covered the consent to the work, the regulation of the arousal and the screening that it requires, the limits of the practitioner and the need for supervision, the transparency of the parental work, the prohibition on the emotional pressure of the practitioner, and the confidentiality of the disclosure of the core emotion.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain the clinical observation that produced the emotion-focused model and the failure of the cognitive therapies it responded to.
2. State the two intellectual sources of the model.
3. Explain the research finding that changed the field.
4. State the four reasons for the model's relatively limited clinical adoption.
5. State the model's foundational claim and explain what it does not mean.
6. State the four components of Greenberg's account of emotion.
7. Explain the needs framework and the clinical question it generates.
8. Distinguish primary from secondary emotions, and explain why the secondary is often the target.
9. Explain emotion schemes and the level at which the model locates the vulnerability.
10. Explain emotion as transcendence and the solution emotions.
11. State the position on the relation between emotion and cognition, and the diagnostic question it generates.
12. Define the change point and explain why the practitioner must find it.
13. State the two components of the change point and the consequence of producing only one.
14. Describe the practitioner's task in reaching the change point.
15. State the five steps of the emotional processing.
16. Describe the four emotion coaching procedures.
17. State the parental emotion coaching hierarchy and identify the two steps parents most often skip.
18. State the two developmental modifications that make the emotion coaching usable with adolescents.
19. State the four reasons the model fits the adolescent period.
20. State the six specific adaptations for young people.
21. Describe the four presentations for which the model is particularly suited.
22. State the three limits of the model with young people.
23. Describe the case of Tia and state the three features of it that generalise.
24. Summarise the status of the evidence and its four limitations.
25. State the five strengths and the five limitations of the model.
26. Explain the four cultural considerations for emotion-focused work.
27. State the correction required by the politics of the model.
28. State the seven ethical requirements for this work.

**Review and Discussion Questions (continued)**

## 14. Practice Activities

**Practice and Application**

### Activity 37.1 — The Secondary Emotion Audit (60 minutes, individual)

Take a real case from placement and write the presenting emotion, the possible core emotion, and the specific secondary emotion that is blocking the core one. Then write the evidence for each: what in the account supports the identification, and what would make you revise it. Then write the appraisal — the meaning of the situation for the core need — in one sentence in the young person's own words. The final step is the one that distinguishes the model from a general empathy exercise, and most students find it the hardest and the most useful.

### Activity 37.2 — The Emotion Coaching Rehearsal (60 minutes, triadic)

One student plays a nine-year-old who is furious because a friend would not share a toy; one plays the mother, who is herself at her limit; one observes. The mother practises the four-step hierarchy: empathise, validate, label and limit, problem-solve. The observer's task is to note which steps were skipped and to note the mother's own emotional state as she attempts each. The last observation is the important one, because it is what Chapter 40 and the parent work of Chapter 33 are for.

### Activity 37.3 — The Change Point Sequence (90 minutes, small group)

In groups of three, take a case and write out the full sequence to the change point: the core emotion, the blockage, the work with the blockage, the approach to the peak, the emotional component, the cognitive component, and the resolution. Then write the specific words the practitioner would use at each step. Then write what the practitioner would do if the young person did not reach the peak. The last part is the practical reality of the work, and it is the part that is least often discussed in the literature.

### Activity 37.4 — The Cultural Display Audit (60 minutes, individual)

Take a young person from a family whose emotional rules differ from your own and write the family's display rules: which emotions may be expressed, to whom, in what form, and what happens when the rules are broken. Then write what this means for the work: what the young person is likely to show in the session, what they are likely to withhold, and what the practitioner would need to establish before the change-point work. Then write the ethical implication. The exercise is the most direct preparation for the cultural work in Chapter 48.

## 15. Annotated Reading Pathways

**For the model.** Read Greenberg and Rice (2001) for the process work and Greenberg and colleagues (2003) for the clinical procedures, and read Greenberg (2015) for the contemporary statement.

**For the emotion coaching.** Read Cooper and Golan (2013) for the adolescent applications and the Gottman material for the parental work. The emotion coaching is the most practically usable element of the model and the easiest to learn.

**For the process research.** Read the studies on in-session emotional change and on the change point. The process research is what justifies the model and it is more convincing than the outcome trials.

**For the wider emotional science.** Read the affective neuroscience literature on the adolescent period and on the developmental trajectory of the emotional system, and read the developmental emotion regulation literature. The model is a clinical model and the science is what tells you when it is safe to use it.

**For the critique.** Read the critical accounts of the emotion-focused model's account of emotion and the evidence limitations, and read the cultural critiques of Western affect models. The model is more useful when its boundaries are known.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Emotion-Focused Therapy** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is greenberg's model of emotion regulation, the change point, and emotion-focused work with youth in distress. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | "This account fits, partly fits, or needs changing." | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | "These goals are mine as well as the service's." | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | "I have a way to begin, not just an instruction." | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | "The work is challenging but understandable and stoppable." | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | "This is not only something I do in a counselling room." | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | "A setback leads to inquiry, not blame." | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | "Adults are changing some conditions too." | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | "I can explain what helps and why." | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | "Ending is planned; I am not disappearing from care." | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.**

Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Amara is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Amara's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Amara explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Amara chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Amara has approved.

At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Amara can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?

- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Emotion-Focused Therapy

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Emotion-Focused Therapy** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Emotion-Focused Therapy** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider

disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

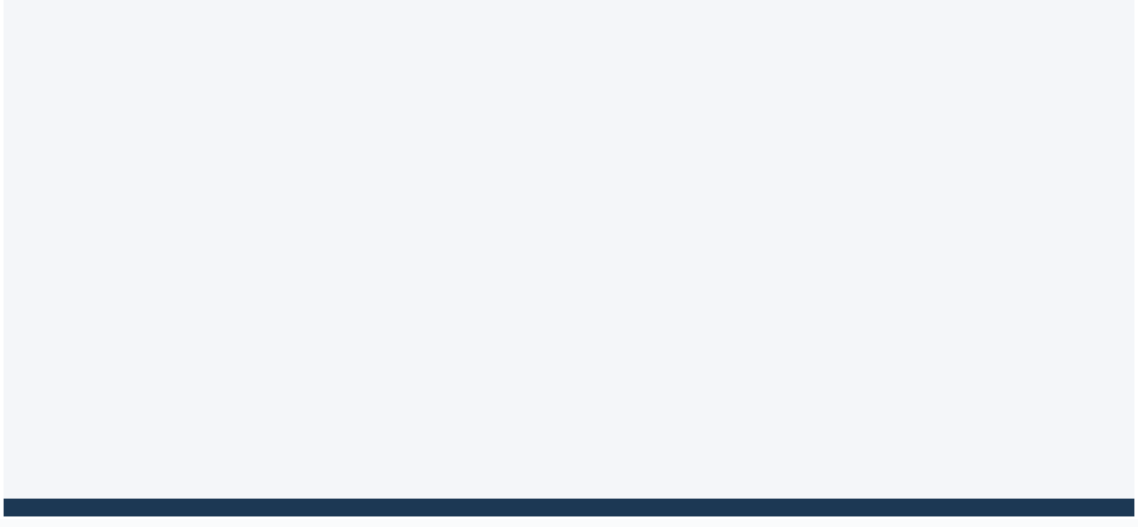
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Emotion-Focused Therapy**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Emotion-Focused Therapy to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 38

# Trauma-Informed Practice: Principles, Phases, and Precautions

The neurobiology of threat, the three-phase model, and the ethical floor rules of trauma work.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the development of the trauma-informed principle and describe the clinical and political context of its emergence.
- State the four core principles of trauma-informed practice and explain the operational meaning of each.
- Describe the threat response system, including the three states and their functions, and explain the clinical consequences of a dysregulated nervous system.
- Describe the three-phase model of trauma treatment and the clinical reasons for the phase structure.
- Apply the safety and the ethical precautions that govern work with a young person with a trauma history.
- Critically evaluate the trauma-informed principle, including the questions of the definition of trauma, of the risk of individualising a structural problem, and of the evidence for the phase model.

## Chapter Overview

The trauma-informed principle is not a therapy. It is a **stance**, and its most consequential feature is that it begins with the question of what has happened to a person rather than with the question of what is wrong with them. This is not a semantic preference. A service that asks what has happened will make different decisions about waiting lists, about the physical environment, about the involvement of a young person in decisions about their own care, about the behaviour of staff, and about the documentation of a young person's history.

The principle emerged from a specific convergence in the 1990s. Three developments made it possible. The **empirical** development was the accumulation of a large research literature on the neurobiology of traumatic stress, on the developmental consequences of chronic threat, and on the effects of adverse childhood experiences, which established beyond reasonable dispute that the experience of threat is itself a health event. The **clinical** development was the recognition that the therapeutic approaches to trauma had been developed for adults with single-incident trauma and were being applied to children with chronic developmental trauma, which is a different condition. The **political** development was the recognition, driven by the survivor movement and by the investigations into institutional treatment, that the systems which were supposed to treat trauma had frequently caused it.

The four principles are **safety**, **trustworthiness and transparency**, **peer support**, and **collaboration and mutuality** — and the fourth of these is the one most often confused with its opposite, because "collaboration and mutuality" means that the work is done *with* the young person, and a great many of the practices that are described as trauma-informed are in fact done *to* the young person.

This chapter presents the principles, the threat response system, the phase model, the precautions, and the critical analysis, which includes the serious question of whether the trauma frame can become a way of locating a young person's difficulties in their own history when the history has been produced by the institutions around them.

## 1. Historical Context

### 1.1 The history of the concept

The idea that a service should be organised around the understanding that many of the people it serves have been traumatised is not new in its content, but it is new in its scope.

The **early** history belongs to the veterans' and the disaster mental health services, in which the observation that the services themselves were retraumatising — through intrusive procedures, through the compulsory and unpredictable treatment, and through the environments — led to a first generation of trauma-informed principles, in the 1980s.

The **second generation** emerged in the 1990s from the children's services and from the foster care and adoption literature, and it was driven by the recognition that the children being placed in care had typically been traumatised by the very systems that then processed them.

The **third generation** is the current one, and it is largely the product of the **National Child Traumatic Stress Network** in the United States, which published a conceptual framework in 2005 and a practitioner toolkit, and which organised the field around the four principles. The framework is now the most widely used definition in the English-speaking world, and it has been adopted in national policy in several countries.

## 1.2 The four R's and the six principles

The National Child Traumatic Stress Network's framework is often summarised in the four R's — **recognise**, **respond**, **refer** — with a fourth, **re-educate**, added by others. The fuller statement is the **six principles** of the SAMHSA framework, which are:

**Safety.** Throughout the organisation, the physical and emotional safety of the people served is prioritised.

**Trustworthiness and transparency.** The organisation is clear about what it will do, and it is honest about what it cannot do.

**Peer support.** The organisation actively supports and links with the peer support available in the community.

**Collaboration and mutuality.** The organisation works with the people it serves, and the people it serves share power in the decisions.

**Cultural, historical, and gender issues.** The organisation recognises and responds to the cultural, historical, and gender issues in the lives of the people it serves.

**Empowerment, voice, and choice.** The organisation honours the voice and the choices of the people it serves.

The four core principles are the first, second, third, and fourth, and the last two are usually treated as cross-cutting. A student should be able to state all six, and should be able to give an organisational example of each, because the principles are properties of the organisation and not only of the practitioner.

## 1.3 The developmental context

The developmental context is what makes the trauma-informed principle different in child and adolescent services than in adult services, and it is essential to understand.

**The developing nervous system is more plastic and more vulnerable.** The adolescent brain is in the middle of the second great period of synaptic reorganisation, and the period is one in which the stress response systems are highly reactive and the prefrontal regulatory systems are not yet fully mature. The

adolescent is therefore more affected by chronic threat and less able to modulate it.

**The "child is not a small adult" principle.** The approach of treating a child as an adult with a smaller body produced the cognitive behavioural therapies of the 1980s and 1990s applied to children, and it is now understood to be an error in the treatment of trauma. A child who has been abused does not have a distorted appraisal of a safe situation; the situation is dangerous.

**Trauma is not a single event but a pattern.** The diagnostic criteria of the DSM and the ICD were designed around the single-incident trauma of the adult, and the developmental trauma of childhood — chronic, relational, repeated, and occurring during development — produces a different presentation, and the "complex trauma" or developmental trauma framework of the 1990s was developed to describe it.

**The developmental impact is cumulative and is mediated by the presence of a safe adult.** The research on the developmental trauma literature has established that the single most important protective factor against the impact of developmental trauma is a consistent, responsive adult in the young person's life, and this is why the work with the caregivers is central to the treatment rather than ancillary to it.

# 2. The Threat Response System

## 2.1 The three states

The clinical vocabulary of trauma-informed practice is most easily taught through the description of the threat response system, and a student should be able to state the three states and their functions.

**The dorsal vagal state**, sometimes described as the immobilised or shut-down state. The functions are conservation and the shutdown of the system when fighting and fleeing are not available or are not effective. Its signs are the ones that are easily mistaken for defiance: the blank stare, the flat affect, the "nothing" answer, the failure to respond, the refusal to move, the dissociation, and in the young person, the appearance of not caring. It is important for a practitioner to know that this state is not a choice and is not an attitude, and that a young person in this state cannot be reasoned with, cannot be challenged, and cannot be engaged in any task.

**The sympathetic state**, the fight-or-flight state. The functions are mobilisation and the rapid preparation for action. Its signs are the increased heart rate, the shallow rapid breathing, the tremor, the increased muscle tone, the narrowed attention, the rapid speech, the raised voice, the agitation, and the inattention to everything that is not the perceived threat.

**The ventral vagal state**, the social engagement state. The functions are the maintenance of safety through connection, the moderation of the other two states, the capacity for calm, and the capacity for social engagement. Its signs are the settled affect, the steady voice, the eye contact, the capacity to receive comfort, and the capacity to think.

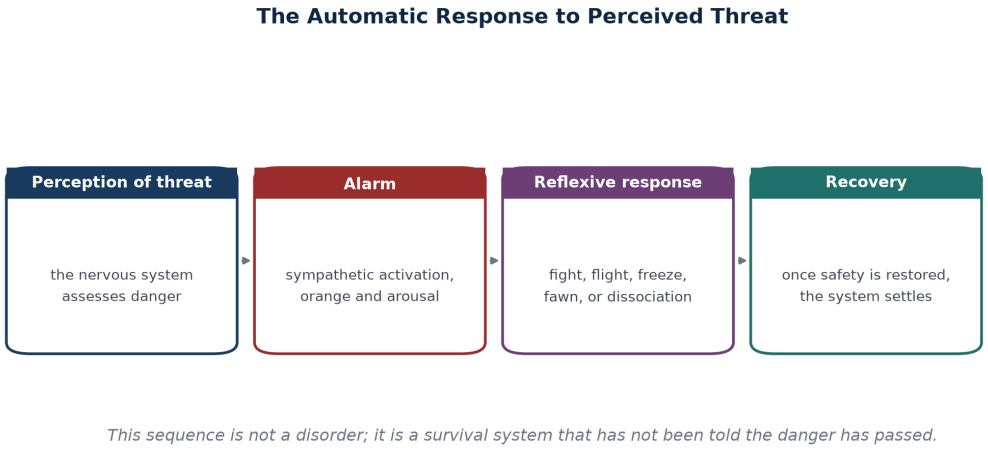



**Figure 38.27** The three states of the threat response system, showing the dorsal vagal, sympathetic, and ventral vagal states, their functions, and the characteristic signs of each.

## 2.2 The importance of the state for practice

The single most practically useful thing a practitioner can know about the threat response system is that **the client's state determines what intervention is possible.** A young person in the dorsal vagal state cannot be engaged in a cognitive task, a young person in the sympathetic state cannot be given an insight, and a young person in the ventral vagal state can do both.

A great many clinical failures with young people with trauma histories are failures of state, not failures of technique. A practitioner who tries to reason with a young person who has gone blank is not being skilful; the technique is unavailable.

The clinical rules that follow are simple and a student should be able to state them.

**If the young person is in the dorsal vagal state, the intervention is orienting and physical.** The voice is low and slow, the language is concrete, the focus is on the external environment, and the goal is to restore the ventral vagal state. Talking about feelings will not work and may make things worse.

**If the young person is in the sympathetic state, the intervention is containment and reduction.** The arousal is reduced before any content is addressed, and the reduction is done through the body — the breathing, the grounding, the movement — because the young person cannot be talked down from a sympathetic state.

**If the young person is in the ventral vagal state, the work can proceed.** This is the state in which the cognitive, the emotional, and the narrative work of this book is possible.

## 2.3 The window of tolerance

The **window of tolerance** is the range of arousal within which a person can function and in which information can be processed. Above it is the hyperaroused state; below it is the hypoaroused state. The developmental and clinical significance is that a young person with a trauma history has a narrow window, that the narrowing is itself a symptom, and that a good deal of what is called "non-compliance" in this population is a young person who has been pushed outside the window.

A practitioner working with a young person with a trauma history should be able to estimate the width of the window and should be working to widen it, because the widening is a precondition for everything else. The practical implication is that the preparatory and stabilisation work of Chapter 40 is not a preliminary to the treatment; it is a substantial part of it.

# 3. The Three-Phase Model

## 3.1 The structure

The three-phase model, which was developed principally from the work on complex trauma with adults and then applied to children and adolescents, divides the treatment into three phases with explicit criteria for movement between them.

**Phase one: safety and stabilisation.** The establishment of safety, in the broadest sense: physical safety, emotional safety, relational safety, and the practical safety of housing, income, and routine. The work is on the reduction of the symptoms, the establishment of the coping and grounding skills, the building of the relational safety with the practitioner, and the resolution of the practical problems that keep the young person in a threat state.

**Phase two: trauma processing.** The deliberate, consented, and carefully paced work on the traumatic memories and on the meanings attached to them. The phase includes the specific trauma-focused interventions — the protocols of Chapter 39 — and the narrative and meaning-making work.

**Phase three: integration and rehabilitation.** The consolidation of the changes, the rebuilding of the young person's life, the re-engagement with education, with relationships, with the future, and the development of the ordinary skills of a life that has been interrupted by the trauma.

## 3.2 The criteria for phase movement

The phase model's most practically useful feature is its explicit criteria for movement, and a student should be able to state them.

**The criteria to move from phase one to phase two** are: the young person is able to remain present in session and is not repeatedly dissociating; the young person can use the grounding and coping skills independently; there is sufficient relational safety in the therapeutic relationship; the practical situation is stable enough that the processing will not be interrupted by a crisis; the young person is able to tolerate the affect associated with the trauma narrative; and the young person has consented to the processing work, informed and ongoing.

**The criteria to move from phase two to phase three** are: the trauma narrative has been processed sufficiently that the memories are no longer intrusive or overwhelming; the young person has made a meaning of the experience that they can live with; the symptoms have reduced; the young person's functioning has improved; and the young person has developed enough coping to manage the ordinary stresses.

**The criteria to move backwards** are as important and are frequently forgotten: a return to phase one when the young person's functioning deteriorates, when the processing is reopened, or when a new crisis occurs.

## 3.3 The criticisms of the phase model

The phase model is the most influential framework in the field and it is also the most criticised, and a student should be able to state the criticisms.

**The evidence for the model as a package is thin.** The phases have not been tested as an integrated protocol in controlled trials. The evidence is for the components: the evidence for the specific trauma-focused interventions is good, and the evidence for the stabilisation skills is more limited.

**The phases can become a way of avoiding the difficult work.** A practitioner who is uncomfortable with the processing phase can keep the young person in phase one indefinitely, and the young person's compliance with the stabilisation work becomes the price of admission.

**The model under-recognises the developmental reality.** The phases presume a capacity for sustained processing that many young people do not have, and the developmental trauma literature argues that a young person's processing may need to be much more gradual and much more embedded in daily life.

**The model's universality is questionable.** The phase model was developed in a particular clinical culture, and the sequence assumes that safety can be established before the trauma is addressed, which is not always true, and which is not true in some cultural and political settings where the safety cannot be established by the individual or the practitioner at all.

## 4. Further Reading for This Chapter

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- Cohen, J. A., Mannarino, A. M., & Deblinger, E. B. (2017). *Treating trauma and traumatic grief in children and adolescents*. — The TF-CBT model.
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- Mensches, S. (2019). *The body remembers*. — The somatic account.

- Treleaven, B. (2018). *Trauma: A practitioner's guide to counselling*. — The somatic clinical practice.
- Shelter from the Storm. — The psychoeducation resources for young people.
- National Children's Alliance. — The child advocacy centres model.
- Moore, C., & McNair, L. (2017). *The practitioner handbook of trauma therapy*. — The phased clinical handbook.

# 5. The Precautions

## 5.1 The ethical floor

The precautions are the practical expression of the ethics of the work, and a student should be able to state them as rules.

**Do not force the disclosure of traumatic material.** The disclosure is not the treatment. The treatment is the young person's capacity to live with their history, and a young person who is pressed to disclose has been given a second experience of powerlessness over the very material that was taken from them. The consent to the processing phase must be explicit, informed, ongoing, and revocable.

**Do not use a trauma-focused intervention with a young person who is not stabilised.** The specific protocols — the EMDR of Chapter 39, the trauma-focused cognitive behavioural therapy — are contraindicated in an unstabled young person, and using them is a technical error with clinical consequences.

**Do not require the young person to remain in a session in which they are outside their window of tolerance.** The session is ended, and the ending is planned rather than improvised, and the young person is not left to manage their own dysregulation alone.

**Do not treat the disclosure of abuse as the goal.** The goal is the young person's safety, their development, and their functioning. A young person who has disclosed and who is safe and who is developing has been successfully treated, whatever they have or have not disclosed.

**Do not use the trauma frame to override the young person's own account of themselves.** A young person who says they are fine and who declines to describe their history has the right to say that, and a practitioner who insists is doing so in the name of the trauma.

**Do not make the disclosure a condition of support.** This is the requirement of the safeguarding framework, and a practitioner must know the local law and the local guidance, and must be clear with the young person at the outset what the limits of confidentiality are.

## 5.2 The clinical precautions

**Do not begin with the trauma narrative.** The first task is the history, the current functioning, the strengths, the resources, and the context, and the trauma narrative is a later task.

**Do not work with the memory before the affect can be tolerated.** A young person who cannot stay present in the room cannot process a memory, and the work on the tolerance comes first.

**Do not assume the timing.** The young person who has disclosed in week two has not begun processing, and the practitioner who mistakes a disclosure for a readiness to process will do harm.

**Do not work with a young person who is still in an unsafe environment.** A young person who is returning to a house in which the abuse continues cannot be in phase two, and the practitioner who works on the memory with such a young person has misunderstood the model.

**Do not neglect the body.** The trauma is in the body, and the somatic work of Chapter 43 is frequently the work that allows the cognitive and the narrative work to proceed.

**Do not neglect the caregiver.** A young person with a trauma history whose caregiver is not able to provide safety will not be helped by the individual work, and this is the most common reason for the failure of phase-based treatment with young people.

### 5.3 The organisational precautions

The trauma-informed principle is a property of the organisation, and a student preparing for practice should know what to look for in a service and what to advocate for.

**The physical environment.** Sight lines, exits, seating that does not put a person with their back to a door, the lighting, the noise, the privacy, and the absence of waiting in a queue with the people who triggered the referral.

**The front door.** Who is greeted, by whom, and what is said. A young person who has been referred by a school and who arrives to be questioned in a waiting room with the school's staff in it has been re-traumatised before the first session.

**The consent and the information.** A young person should be told, before the first session and in writing, what will happen, what will be confidential, what the limits are, and what their choices are.

**The staffing and the supervision.** A service that works with this population and does not have specific supervision of the staff is exposing the staff.

**The language of the organisation.** The language of the service's documents, of its letters to parents, and of its reports to a school is a trauma-informed practice question, and the language of a report about a young person who has disclosed abuse can determine the young person's future.

**The avoidance of retraumatisation in the systems.** A young person with a trauma history who is moved between four different professionals in a year has been subjected to a form of retraumatisation, and continuity is a clinical intervention.

## 6. Trauma-Informed Practice with Children and Adolescents

### 6.1 The developmental modifications

**Safety is established through the body before it is established through the relationship.** A young person who has been abused by adults will not feel safe in a room with an adult in it, however warm the adult, and the safety that is achieved first is the physical safety: the predictable routine, the door that is not locked, the knowledge of where the exits are, the reliability of the times, and the small, kept promises.

**The relationship is the treatment and it takes longer.** The relational safety with a young person with a developmental trauma history is not established in the first month, and the practitioner should be prepared for a period in which the young person is polite and formal and does not bring anything. The period is not a

failure of rapport; it is the young person testing, and the test is over months.

**The disclosure is a clinical event, not a milestone.** A disclosure should be met with a predictable, calm, and brief response, and the practitioner should have prepared what they will say in advance, because the young person will be watching the practitioner's face.

**The developmental impact is the target, not the memory.** A young person does not need to remember the event in order to be affected by it, and the treatment of the developmental consequences — the regulation, the attachment, the self-concept, the developmental tasks — is often the more important work.

**The work with the caregivers is central.** Where a young person remains with a caregiver, the caregiver's capacity to provide safety is the determinant of the outcome, and the practitioner who works only with the young person is working against the grain.

**The work is developmental in its tasks.** The trauma work with an adolescent includes the developmental tasks of Part V: the differentiation, the peer relationships, the identity, the academic engagement, and the future. A young person whose trauma is processed and who is then not helped to build a life will not stay well.

## 6.2 The presentations

**The young person with a chronic somatic presentation.** The somatic work of Chapter 43 is the entry point, and the somatic presentation is often the most legitimate and least frightening route into the treatment.

**The young person with a dissociation presentation.** The dorsal vagal state is the clinical fact, and the practitioner should be able to recognise it and should not attempt to work with the content while the young person is in it.

**The young person with an attachment-based presentation.** The attachment work of Chapter 34 and the mentalisation work of Chapter 42 are the natural approaches, and both are compatible with the trauma work.

**The young person with an eating presentation.** The family-based approach and the emotion-focused work of Chapter 37 are both relevant, and the somatic and the trauma work may be necessary.

**The young person in a care placement.** The placement is the trauma, and the clinical task is the stability of the placement and the availability of a consistent adult.

## 6.3 The specific working with young people who have been sexually abused

This presentation requires its own rules, and a student should know them.

**The assessment of the safety is the first and the most important task**, and it is ongoing.

**The parent's role must be established explicitly**, because the parent may be the perpetrator, and the practitioner must know which before the work begins.

**The do-not-contact orders and the supervision plans** are the framework in which the work is done, and the practitioner must understand them.

**The young person's own account of the abuse is not the only evidence and does not need to be complete.** The clinical work does not require the young person to have told the story in detail, and the pressure to disclose a detailed narrative is a pressure that the ethical frameworks explicitly reject.

**The behavioural manifestations must not be over-interpreted.** A young person who is hypervigilant, who is intrusive, who has sexualised behaviour beyond what is age-typical, who is oppositional, or who is enuretic, is presenting a set of responses to an abnormal environment, and the responses are not evidence of the young person's character.

**The father's role** requires particular care, and the professional position is that the involvement of a non-offending parent is generally beneficial and the exclusion of a non-offending parent is generally harmful, subject to the specific facts of the case and the local law.

## 7. Case Illustration: The Boy Who Could Not Be in the Room

**Presentation.** Samuel is twelve and is referred by a school safeguarding lead after a disclosure made to a teaching assistant about a stepfather. He has not been in school for eleven weeks. The referral states that he is "non-compliant with school attendance" and that his mother is "not engaging".

**The first contact.** The practitioner does not make the first contact by telephone with the mother. The first contact is a letter to the mother, and the letter says what the practitioner can and cannot do, and it says that the practitioner will also be writing to Samuel, and that the practitioner would like to speak with Samuel directly and separately. This is a standard and important procedural point: the young person is contacted directly and is not dependent on the adult who may be implicated.

**The direct contact.** Samuel does not respond to the letter, and the practitioner does not press. Three weeks later, a letter arrives, handwritten, containing four words: "I don't want to talk." The practitioner treats this as communication rather than as refusal, and the reply is short: "That's okay. I won't ask you to talk about anything you don't want to. I would like to meet you somewhere you feel okay, and to know what would help. You can reply to this letter with anything, or with nothing."

**The first meeting.** Samuel comes, with his mother, and the practitioner arranges the meeting in a room that is not the clinical room: a small room in the community centre with a window and a door that is not locked, and two chairs at an angle rather than across a desk. The practitioner says at the outset: "I am not going to ask you about your stepfather. Not today and not unless you want to. I am going to ask you about your week, and about what would make school better, and about what you need. You can stop at any time, and we can stop the whole thing, and if you stop, that is not a problem."

Samuel's dorsal vagal state is evident: he is flat, he is monosyllabic, and he does not look at the practitioner. The practitioner does not attempt rapport. She asks about the walk to the community centre, and the answer is that the walk is twenty minutes and he comes alone. The practitioner asks what he sees on the walk, and he names three specific things. The work in this session is entirely orienting, and the practitioner does not attempt any of it, and the session is twelve minutes long, and it ends with a specific arrangement: the same time and the same place next week.

**The phase one work.** The next three months are phase one, and the work is unglamorous and is the work that makes everything else possible.

**The physical safety.** A written safety plan, with the specific details of who Samuel lives with, when he is with whom, and what happens if his stepfather is in the house. The plan is shared with the safeguarding lead and with the mother's social worker, and the practitioner asks the mother directly, in a way that is careful and clear, whether Samuel's stepfather lives in the home. The mother says that he does not, and the

practitioner asks what the arrangement is, and the mother says that he comes on Sundays, and the practitioner asks whether Samuel is ever in the house on Sundays, and the mother says that Samuel stays at his grandmother's on Sundays, and the practitioner asks who arranges that, and the mother says that she does. The practitioner makes no judgement and records the account and continues.

**The practical problems.** The school attendance. Samuel has not been in school for eleven weeks and the referral calls this non-compliance. The practitioner asks what school was like before the referral and finds that Samuel was a good attender until the week of the disclosure, and that the school has not contacted him since. The school has done nothing. The practitioner discovers that the form tutor did not know what had happened and did not know that Samuel was distressed, and the practitioner makes the case to the school for a reintegration plan with a named adult, and the school agrees, and the first step is that Samuel comes to the school for one period with his form tutor, and the school is told that the "non-compliance" label is to be removed from the record and replaced with "ceased attendance following a safeguarding referral", and the school agrees.

**The grounding and the stabilisation.** The grounding work of Chapter 40 is taught explicitly, in the sessions, and it is taught as a set of practical skills for a twelve-year-old. The emotional regulation skills are taught before the disclosure work and are taught to the mother as well.

**The relational work.** The sessions are the same time and the same place, and the practitioner reliably turns up, and for six weeks Samuel says almost nothing beyond single words, and the practitioner does not fill the silence and does not comment on it. In the seventh week Samuel says, without preamble: "There's a thing on Sundays." The practitioner says: "Okay." And he tells the practitioner, over the following four sessions, what happens on Sundays, and the practitioner does not stop him, and the practitioner does not ask questions, and when he finishes the practitioner says: "Thank you for telling me. That was hard to say." And nothing else.

**The work with the mother.** The practitioner's work with the mother is the necessary work, and it is the work that most affects the outcome. The mother is shown the "what is your child worried about" material, and she is taught the grounding skills, and the practitioner asks her the question that the whole case turns on: "What did you think was happening on Sundays?" And the mother says, after a long silence: "I thought he was just being difficult." And the practitioner says: "He was telling you something. He was trying to get you to notice." And the mother cries, and the practitioner stays with her, and the mother's relationship with the safeguarding system — and with the practitioners who have been involved — is repaired a little, and the mother's participation in the plan changes entirely.

**The outcome.** At six months, Samuel is back in school four days a week, has a named adult at school, is seeing his grandmother on alternate Sundays with his mother, and has a written safety plan that he helped to write. The stepfather has not had contact with him. The mother is engaged with the social worker and with the practitioner. The trauma processing work has not yet begun, and the practitioner says clearly that it may not be needed in the form the phase model assumes, and that the work has been the building of a life that is not organised around the secret.

**The interpretation.** The case illustrates the trauma-informed principle in its pure form. Nothing in it was a trauma-processing intervention. The interventions were: the direct contact with the young person; the removal of the "non-compliant" label from the record; the addressing of the school's failure to re-engage him; the explicit safety assessment; the building of a predictable, bodily, non-verbal relational safety; the work with the mother, which was the decisive intervention; and the careful, unhurried reception of the disclosure when it came. The young person was never asked to talk before he wanted to, and the disclosure

that eventually came was the result of eleven weeks of predictability, not of a technique.

# 8. Evidence and Research Support

## 8.1 The status of the evidence

**The evidence for the individual components is good.** The specific trauma-focused protocols — the trauma-focused cognitive behavioural therapy and the EMDR — have strong evidence bases, and the first is the best-evidenced treatment for the trauma symptoms of children and adolescents.

**The evidence for the phase model as an integrated package is thin**, and a student should be candid about this. The phases have not been tested as a package in controlled trials.

**The evidence for the stabilisation skills is more limited and more developmental.** The grounding and the stabilisation work of Chapter 40 has a smaller and more recently developed evidence base, and the strongest evidence is for the general principle that the reduction of arousal precedes the cognitive work.

**The evidence for the trauma-informed service-level reforms is largely indirect**, and the field has relied on the plausibility of the principles, on the plausibility of the mechanisms, and on the consistent finding that the reforms improve the experience of service users and the well-being of the staff.

**The evidence for the principles themselves is now much stronger than it was.** The large studies of the health and educational outcomes of adversity, and of the dose-response relationships between the number of adverse experiences and the outcomes, and of the protective effect of a stable adult, have given the principles their epidemiological foundation, and a student should know the shape of this evidence even though the principles are not themselves interventions.

**The evidence for the specific model of the Attachment and Biobehavioral Catch-up and the CFTSI is good**, and the video-feedback mechanism is understood.

## 8.2 Limitations of the evidence

**The heterogeneity of the interventions makes the meta-analytic findings difficult to interpret**, and the pooled effects conceal substantial variation.

**The samples are dominated by the children of the adoptive or foster care system**, and the young people who are in the general population and who have been exposed to community violence, to domestic violence at a level that does not trigger a referral, and to the loss of a parent, are under-represented.

**The outcome measures are almost entirely symptom measures**, and the developmental outcomes, the educational outcomes, and the quality of life outcomes are much less well documented.

**The fidelity of the phased delivery is rarely measured**, and this is a significant weakness given that the phases are the model's principal mechanism.

**The long-term follow-up is limited**, and the developmental question — whether the gains are maintained into the adult years — is largely unanswered.

# 9. Strengths and Limitations of Trauma-Informed Practice

### 9.1 Strengths

**It starts with what happened.** The reorientation from the disorder to the history is the most important single change in the field's practice in forty years.

**It is developmentally sound.** The child-is-not-a-small-adult principle has corrected a substantial error.

**It has an evidence base of the right kind.** The epidemiological evidence is strong and the intervention evidence is good for the components.

**It is compatible with every other approach in this book.** A stabilisation phase is compatible with cognitive therapy, with the behavioural work, with the attachment work, with the mentalisation work, and with the somatic work.

**It has changed services.** The principle has influenced the physical environment, the front door, the consent procedures, and the language of the documentation, and these are not trivial changes.

**It has changed the professional self-conception.** The practitioner's role in this model is not primarily to interpret but to support.

### 9.2 Limitations

**It can individualise a structural problem.** This is the most serious criticism and it is addressed in Section 10.

**The definition of trauma is broad and contested**, and a framework that includes everything risks including nothing.

**The phase model can become a way of avoiding the work**, and the young person can be kept in phase one indefinitely.

**The model presupposes that safety can be established**, and in some circumstances it cannot, and the practitioner is then left without a model.

**The culture of the field has become strongly trauma-informed without a corresponding evidence base for many of its specific practices**, and the enthusiasm has sometimes outrun the technique.

**The risk of the "trauma narrative" becoming the young person's identity.** A young person who has been told for two years that they are a trauma survivor and who has been asked to tell the story repeatedly can acquire an identity organised entirely around the trauma, and this is the same error as the diagnostic identity and it is less visible.

## 10. Cultural and Ethical Considerations

### 10.1 The individualisation of a structural problem

The most serious ethical danger of the trauma-informed principle is that it locates the young person's difficulties in their history, and that this can be used, and is sometimes used, to explain away the conditions in which they live.

A young person who is in an unsafe school, who is in temporary accommodation, whose family is in poverty, who is being treated as a problem by the adults around them, and who has a history of a difficult event, does not need the history to be addressed first. The history is the reason the conditions are what they are, and the conditions are what the young person is currently living in, and the treatment of the conditions

is the treatment.

Three safeguards follow.

**Always ask what the young person is living in.** The question "what is happening in your life right now" is as important as "what happened to you", and it is more often actionable.

**Never let the trauma frame substitute for the structural one.** If the young person's difficulty is a product of the conditions, the conditions must be changed, and the frame of the work is a fight about those conditions and not a journey into the past.

**Keep the history in its place.** The trauma-informed principle does not require the trauma to be the centre of the treatment. In many cases the best work is the establishment of a good enough life in the present, and the young person may never need to process the memory at all.

## 10.2 Culture, trauma, and the meaning of the event

**The meaning of an event is culturally constituted.** The same event — a parent's violence, a community's response to a young person's behaviour, a sibling's death, a migration — means different things in different contexts, and a practitioner who applies a universal account of the traumatic event will misdescribe the young person's experience.

**Some events are not traumatic for the young person who experienced them.** The field's framing requires that the impact be assessed in the individual, and a young person who experienced a frightening event and was not frightened by it should not be given a trauma narrative to inhabit.

**The "trauma" of a community and a culture is a different order of thing** from the trauma of an individual, and the collective, historical, and political injuries of a community are not adequately addressed by an individualised model, and a young person whose community has been subjected to violence may be carrying a collective trauma that requires a collective response.

**Culturally specific healing practices** are frequently more available to a young person and more effective than the individual trauma work, and the practitioner should ask about them before assuming that the individual work is the route.

## 10.3 Ethical requirements

**Informed, ongoing, and revocable consent** to each phase and to each intervention within it.

**The young person as the decision-maker.** The principles require the voice and the choice of the young person, and this must be real rather than nominal, and the practitioner must be willing to accept a decision they disagree with.

**The avoidance of compulsory disclosure.** A young person who declines to disclose has the right to do so and has been treated well.

**The limits of confidentiality stated at the outset**, in writing, and the young person must know before they disclose what will happen.

**The avoidance of retraumatisation by the systems**, including the avoidance of unnecessary assessments, of repeated accounts, of unnecessary moves between professionals, and of intrusive procedures.

**The avoidance of the use of the trauma identity.** The young person's account of themselves is not to be replaced by a professional narrative about them.

**Cultural and historical humility.** The practitioner acknowledges the limits of their own framework and the existence of other frameworks in the young person's life.

**The obligations of the professional to the staff.** A service that works with this population has an obligation to supervise the staff and to care for their own dysregulation, and the neglect of this is a clinical error with clinical consequences.

## 11. Key Terms

**Key Terms at a Glance**

- **Trauma-informed practice** — A stance in which a service is organised around the understanding that many of the people it serves have been traumatised, and in which the question of what has happened precedes the question of what is wrong.
- **The four core principles** — Safety, trustworthiness and transparency, peer support, and collaboration and mutuality.
- **The six principles of the SAMHSA framework** — The four core principles plus the recognition of cultural, historical, and gender issues and the commitment to empowerment, voice, and choice.
- **The four R's** — Recognise, respond, refer, and re-educate.
- **Trauma-informed organisation** — The service-level reforms in the environment, the front door, the consent procedures, the staffing, and the language of the documentation.
- **Dorsal vagal state** — The immobilised or shut-down state, whose signs are easily mistaken for defiance and in which no cognitive or emotional work is possible.
- **Sympathetic state** — The fight-or-flight state, in which the arousal must be reduced before any content is addressed.
- **Ventral vagal state** — The social engagement state, in which calm and connection are possible and in which the cognitive, emotional, and narrative work can proceed.
- **Window of tolerance** — The range of arousal within which a person can function and in which information can be processed; above it is hyperarousal and below it is hypoarousal.
- **The narrowing of the window** — The developmental and clinical consequence of chronic threat, and the reason that the preparatory work is a precondition for the rest.
- **Developmentally traumatised** — The presentation produced by chronic, relational, repeated threat during development, as distinct from a single-incident trauma.
- **The three-phase model** — Safety and stabilisation, trauma processing, and integration and rehabilitation.
- **Phase one** — The establishment of safety in its broadest sense, the reduction of symptoms, the teaching of coping and grounding, the building of relational safety, and the resolution of practical problems.
- **Phase two** — The deliberate, consented, and carefully paced work on the traumatic memories and their meanings.
- **Phase three** — The consolidation of the changes, the rebuilding of the young person's life, and the re-engagement with education, relationships, and the future.

**Key Terms at a Glance (continued)**

- **The criteria for phase movement** — The explicit conditions for moving forward and for moving back, and the practical value of the backward criteria.
- **The ethical floor rules** — The prohibitions on forcing disclosure, on using a trauma-focused intervention with an unstabled young person, on requiring a young person to remain outside their window of tolerance, on treating the disclosure as the goal, and on overriding the young person's own account.
- **The single-incident and developmental trauma distinction** — The difference between the adult single-event model and the pattern that produces a different presentation in a child.
- **The developmental trauma criteria** — The criteria proposed for the developmental trauma category, and the debate about whether they belong in a diagnostic manual.
- **The dose-response finding** — The epidemiological finding that the number of adverse experiences is related to the range of health and educational outcomes, and that the relationship is graded.
- **The stable adult as a protective factor** — The single most important protective factor against the impact of developmental trauma, and the reason the work with the caregivers is central.
- **Retraumatisation by the systems** — The retraumatisation that occurs through repeated assessments, compulsory and repeated accounts, intrusive procedures, and unnecessary moves between professionals.
- **The trauma identity** — The risk that a young person who has been told repeatedly that they are a trauma survivor acquires an identity organised entirely around the trauma.
- **Collective trauma** — The historical, political, and communal injuries that an individualised model does not adequately address.
- **The practitioner as a container** — The role of the practitioner in holding the young person's arousal and affect, and the supervision and self-care that the role requires.
- **The five ways of noticing** — The clinician's attentional practice of noticing the young person's state before noticing their content.

## 12. Chapter Summary

Trauma-informed practice is a stance rather than a therapy, and its most consequential feature is that it begins with what has happened to a person rather than with what is wrong with them — a difference that changes decisions about waiting lists, physical environments, consent, staff behaviour, and documentation. The chapter traced the three developments that made the principle possible: the neurobiological and epidemiological research, the recognition that the adult-derived trauma treatments had been misapplied to children, and the political recognition that the systems treating trauma had frequently caused it.

The four core principles and the six-point framework were stated, with the observation that the fourth, collaboration and mutuality, is the one most often confused with its opposite, because much of what is described as trauma-informed is done to the young person rather than with them. The developmental context was developed: the plasticity and the vulnerability of the developing nervous system, the "child is not a

small adult" principle, the distinction between single-incident and developmental trauma, and the cumulative and mediated nature of the developmental impact.

The threat response system was described with the three states and their signs, and the clinically decisive point was made: the client's state determines what intervention is possible, and a great many clinical failures with this population are failures of state rather than of technique. The three clinical rules were stated for the dorsal vagal, the sympathetic, and the ventral vagal states, and the window of tolerance was described with the observation that the preparatory work of Chapter 40 is a substantial part of the treatment rather than a preliminary to it.

The three-phase model was described with its structure, its explicit criteria for movement in both directions, and its four criticisms, including the criticism that the phases can become a way of avoiding the difficult work and that the model presupposes a safety that cannot always be established.

The precautions were organised into the ethical floor, the clinical precautions, and the organisational precautions, and the organisational section is unusual in a chapter of this book in treating the principle as a property of the organisation rather than of the practitioner. The developmental modifications were described, with the direct contact with the young person and the parent's role as the two that are frequently got wrong, and the specific working with young people who have been sexually abused was treated in its own subsection with its own rules.

The case of Samuel demonstrated the principle in its pure form, and the practitioner never conducted a trauma-processing intervention. The interventions were the direct contact without the implicated adult as intermediary, the removal of the "non-compliant" label from the school record, the school re-engagement, the explicit safety assessment, the twelve-minute orienting first meeting, the grounding work, the unhurried reception of the disclosure when it came, and the work with the mother — which was the decisive intervention, and which was turned by a single question about what the mother thought had been happening on Sundays.

The evidence was presented with its structure: good evidence for the individual components, thin evidence for the phase model as a package, more limited evidence for the stabilisation skills, largely indirect evidence for the service-level reforms, and strong epidemiological evidence for the principles. The limitations were named.

The strengths and limitations were set out, with the individualisation of a structural problem identified as the most serious, and the trauma identity as the error that is less visible than the diagnostic identity and the same in kind.

The cultural analysis addressed the culturally constituted meaning of the event, the young person for whom the event was not traumatic, the collective and historical injuries that an individualised model does not reach, and the culturally specific healing practices that may be more available and more effective. The ethical requirements covered the consent, the young person as decision-maker, the prohibition of compulsory disclosure, the limits of confidentiality stated in advance, the avoidance of retraumatisation by the systems, the avoidance of the trauma identity, cultural and historical humility, and the obligations of the professional to the staff.

## 13. Review and Discussion Questions

### Review and Discussion Questions

1. Explain the three developments that made the trauma-informed principle possible.
2. State the four core principles and explain the operational meaning of each.
3. State the six principles of the SAMHSA framework.
4. Explain why "collaboration and mutuality" is the principle most often confused with its opposite.
5. Explain the developmental reasons why the trauma-informed principle differs in child and adolescent services.
6. State the "child is not a small adult" principle and the error it corrected.
7. Distinguish single-incident from developmental trauma and state the clinical consequence of the distinction.
8. Describe the three states of the threat response system and the signs of each.
9. Explain why the state determines the possible intervention, and state the three clinical rules.
10. Define the window of tolerance and explain its clinical significance.
11. Describe the three phases of the trauma treatment model and the content of each.
12. State the criteria for moving from phase one to phase two, and the criteria for moving backwards.
13. State the four criticisms of the phase model.
14. State the five ethical floor rules.
15. State the five clinical precautions.
16. State the six organisational precautions and explain why the principle is a property of the organisation.
17. State the six developmental modifications for work with young people.
18. State the rules for work with young people who have been sexually abused.
19. Describe the case of Samuel and state the four features of it that generalise.
20. Summarise the status of the evidence and its five limitations.
21. State the six strengths and the six limitations of the principle.
22. Explain the individualisation of a structural problem and the three safeguards.
23. Explain the three cultural considerations.
24. State the nine ethical requirements for trauma-informed work.

## 14. Practice Activities

### Practice and Application

### Activity 38.1 — The State Audit (90 minutes, field-based)

In a placement, observe a young person in three different settings and record, for each, their state: dorsal vagal, sympathetic, or ventral vagal. Record the physical evidence. Then record what the adults in each setting did in response. Most students find that the adults respond to the dorsal vagal state with the interventions that are appropriate to the sympathetic state, and the exercise is a more effective argument for the three-state model than any reading.

### Activity 38.2 — The Phase Placement (75 minutes, individual)

Take a case from placement and place the young person in one of the three phases, with the specific evidence for the placement. Then write the criteria: what would have to be true for them to move forward, and what would have to be true for them to move back. Then write the specific intervention that is indicated *now* and only now. The final step is the discipline of the model and it is the step that prevents the phases becoming a way of avoiding the work.

### Activity 38.3 — The Consent Conversation (90 minutes, pairs)

One student plays a young person who has disclosed and who is frightened of what will happen next; the other plays the practitioner. The practitioner must explain, in plain language, what will happen, what will be shared, with whom, and what the young person's choices are. Then reverse roles. The exercise is difficult and the difficulty is the point: most practitioners over-explain and under-inform, and the young person usually understands the first attempt better than the practitioner expects.

### Activity 38.4 — The Structural Audit (60 minutes, individual)

Take a case with a trauma history and write two accounts. The first is the trauma account: what happened, and what it produced. The second is the structural account: the housing, the income, the school, the family's circumstances, the community, the availability of a safe adult. Then write what proportion of the young person's current difficulty the two accounts explain, and write the interventions that follow from each. Then write the priority. The exercise is the most important one in the chapter and the one most often skipped in practice.

## 15. Annotated Reading Pathways

**For the principles.** Read the National Child Traumatic Stress Network's conceptual paper and the SAMHSA framework, and read the National Child Traumatic Stress Network's practitioner guide. These are short and they are the definitional documents.

**For the science.** Read van der Kolk (2014) for the neurobiological account, Porges (2011) for the polyvagal framework, and Siegel (2010) for the attachment and regulatory account. The three together are the theoretical basis of the practice.

**For the clinical model.** Read the Trauma-Focused Cognitive Behavioral Therapy manual and the EMDR manual, and read the child and family traumatic stress intervention manual. These are the specific protocols and they are more useful than the general accounts.

**For the developmental adaptation.** Read the developmental trauma literature and the work on the child-is-not-a-small-adult principle, and read the child advocacy centre model, which is a service-level application.

**For the critique.** Read the critical literature on the individualisation of structural problems, on the expansion of the definition of trauma, and on the trauma identity. The field's own self-criticism is essential and it is developing.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Trauma-Informed Practice: Principles, Phases, and Precautions** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the neurobiology of threat, the three-phase model, and the ethical floor rules of trauma work. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Jonas is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Jonas's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Jonas explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Jonas chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Jonas has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Jonas can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Trauma-Informed Practice: Principles, Phases, and Precautions

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Trauma-Informed Practice: Principles, Phases, and Precautions** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Trauma-Informed Practice: Principles, Phases, and Precautions** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Trauma-Informed Practice: Principles, Phases, and Precautions**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a

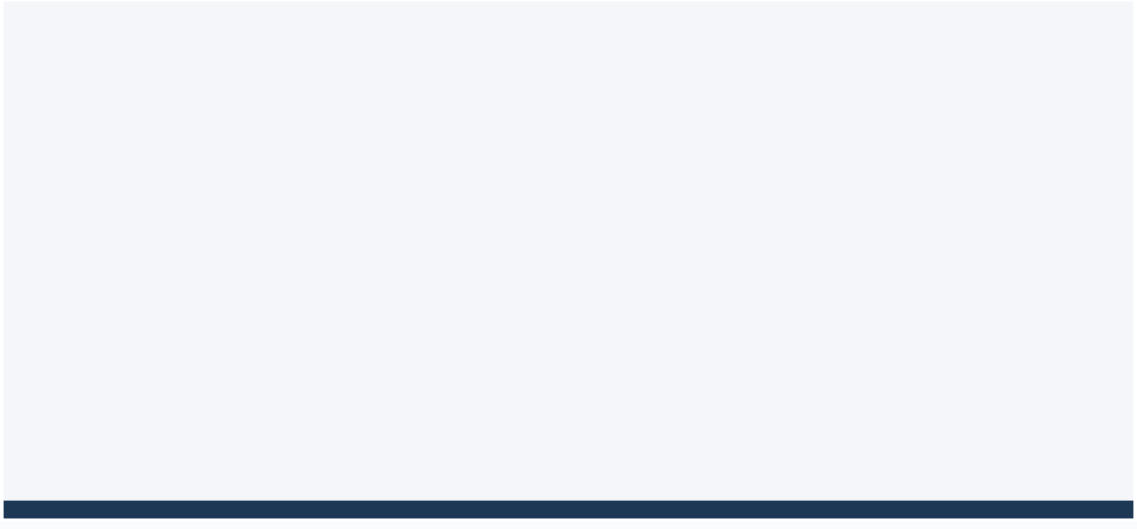
discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Trauma-Informed Practice: Principles, Phases, and Precautions to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 39

# EMDR and Bilateral Stimulation

Shapiro's protocol, the adaptive information processing model, and careful adaptation for children.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the history of the eye movement desensitisation procedure and the development of EMDR, and state the evidence that led to the method's acceptance.
- State the adaptive information processing model and explain how it accounts for the efficacy of a procedure that was originally thought to be effective because of the eye movements.
- Describe the eight phases of the EMDR protocol and the bilateral stimulation methods.
- Explain the mechanisms proposed for the effect of bilateral stimulation, and evaluate the evidence for each.
- Describe the adaptations of EMDR for children and adolescents, including the specific modifications and the evidence for them.
- Critically evaluate the cultural and ethical dimensions of EMDR practice, including the questions of the acceptability of the procedure and of the treatment of the memory.

## Chapter Overview

In 1987, a psychotherapist working with a woman who was suffering from a disturbing memory noticed that she reported a reduction in the distress associated with the memory while moving her eyes from side to side. The observation was made with one patient, it was not a controlled experiment, and it was made in a context in which the prevailing assumption was that eye movements had no psychological effect whatsoever.

Twenty years later, the procedure was a first-line treatment for post-traumatic stress disorder in adults, it was endorsed in the guidelines of the major international bodies, and the underlying mechanism was the subject of one of the more consequential scientific controversies in the history of psychotherapy.

The story is worth telling carefully, because it illustrates a methodological point that the practitioner should take seriously: the eye movements were almost certainly not doing the therapeutic work, and the treatment is effective anyway. The contemporary understanding is that the bilateral stimulation is a **provocative technique** — an apparatus that reliably elicits and sustains the dual attention and the internal association that the treatment requires, and that the same work can be achieved by other means. The evidence for this has been gathered by a series of well-controlled studies comparing eye movements with other forms of bilateral stimulation and with a matched control condition, and the results are more consistent than the field's earlier polemics suggested.

For work with young people, the most important practical facts are these. The procedure with children is markedly different from the adult procedure, and the differences are not cosmetic. The **assessment and preparation phases are longer**, the young person is taught the technique themselves, the clinician is far more active, the **parent is in the room**, and the **body and the regulation work of Chapter 40 comes first**. The evidence for the paediatric protocol is good, and it is better than the evidence for most of the treatments in this book in the specific population with a specific disorder.

## 1. Historical Context

### 1.1 The origin

Francine Shapiro was working with a client who described a memory of an assault that she found unutterably distressing. In the course of the session, the client spontaneously moved her eyes from side to side, and reported, with some surprise, that the memory had become less vivid and less disturbing. Shapiro asked the client to do the eye movements deliberately, and observed a repeat of the effect.

Shapiro then developed the procedure as a systematic treatment, and the early papers described it in terms of a new kind of "information processing" in which the eye movements appeared to facilitate the "unblocking" of traumatic memories. The early formulations were not well specified, and the early trials were criticised, sometimes severely, for their methodology.

## 1.2 The controversy

The controversy was of a familiar and familiar-seeming kind. The critics' arguments were that the eye movements had no established physiological effect, that the method had been given a mystified theoretical justification, and that the early trials had used inappropriate controls. The proponents' arguments were that the empirical results were better than those of the competing treatments and that a mechanism was not a precondition of efficacy.

The field resolved the controversy in a way that is instructive. A series of well-controlled studies established several things: the **efficacy** of the procedure, which was not seriously in dispute even at the height of the controversy; the **specificity** of the bilateral stimulation, in that the procedure is more effective than a matched control that omits the eye movements; and the **absence of a specific mechanism**, in that the effect is found with several different forms of bilateral stimulation and with non-bilateral controls in some studies, which suggests that the stimulation functions as a device for sustaining a particular attentional state rather than as a treatment in its own right.

## 1.3 From eye movements to EMDR

The procedure was renamed and the theory was revised, and the shift is significant. The current model is the **adaptive information processing model**, which locates the pathology not in the memory but in the *failure to process* the memory, and the treatment in the *completion* of the processing. The bilateral stimulation is now understood as the element that elicits and sustains the associative processing while the memory is held in focus.

The renaming was not merely cosmetic. The change in the theory is a change in what the practitioner is doing: the clinician is not moving information out of the memory but is facilitating the patient's own processing of a memory that has not been completed.

## 1.4 The spread

The spread of EMDR into clinical practice has been remarkable, and it has been driven less by the mechanism debate than by three things: the treatment is brief, the guidelines recommend it, and it works. It is now used for post-traumatic stress disorder, for the phobias, for the obsessive-compulsive disorder, for the panic disorder, for the specific phobias, for the grief, and — increasingly, in the paediatric literature — for the specific procedural and accidental trauma of childhood, which is a considerably better fit for the paediatric protocol than the developmental trauma is.

# 2. The Adaptive Information Processing Model

## 2.1 The central claim

The model's claim is that **memory, once encoded and integrated into the general network of autobiographical knowledge, is permanently available for use and for change, and that many pathological conditions are the result of a failure of this processing**.

The implication is that the pathology is not in the content of the memory. The memory is accurate; it happened; the young person was in a car accident and the memory of the car accident is correct. The pathology is in the *storage* of the memory — in the fact that it has remained in a form in which it is experienced as a current event, in which it is accompanied by the physiological response that accompanied the original event, and in which it is not linked into the network of the rest of the person's life.

## 2.2 The components of the model

**The unprocessed memory.** A memory that has been encoded and stored in a form that remains vivid, that is experienced in the present tense, that is associated with the original autonomic and muscular response, and that is disconnected from the general narrative of the person's life.

**The dysfunction.** The symptoms are the consequence of the unprocessed memory: the intrusive images, the nightmares, the avoidance, the hyperarousal, the negative beliefs about the self and the world that the memory has generalised into.

**The link.** The missing element is the *link* between the unprocessed memory and the rest of the person's autobiographical knowledge. When the memory has been linked — when the young person can place it in a narrative of a life in which the accident is one event among many, in which it is over, and in which it is not the whole of who they are — the symptoms reduce.

**The bilateral stimulation.** The device that elicits and sustains the associative processing while the memory is held in focus.

## 2.3 The model and the treatment's logic

The treatment follows directly. The young person is asked to hold the image of the memory in focus while the bilateral stimulation is delivered, and to follow the eye movements wherever they lead, and to report whatever comes. Over the course of the processing, the memory changes: it becomes less vivid, the autonomic response diminishes, and the young person begins to report new associations, some of them painful and some of them neutral and some of them about the present rather than the past.

The final stage, the **installation** of the new associations, is the point at which the change becomes durable, and the installation is done by the clinician rather than by the young person, and it is a distinctive and often under-taught element of the procedure.

# 3. The Eight Phases

## 3.1 The phases

The protocol has eight phases, and a student should be able to name all eight and say what each is for.

**Phase one: the history and the stabilisation.** A full history, including the trauma history, the current symptoms, the medical and the psychiatric history, the developmental and the social history, the current resources, and the current level of functioning. The purpose is to establish that the young person is sufficiently stabilised to proceed and to establish the baseline against which the change will be assessed. A student should be able to state the criteria in Chapter 38 for this phase, because the EMDR history phase is a phase one in the phase model.

**Phase two: the preparation.** The preparation of the young person for the procedure itself: the explanation of what will happen, the establishment of the bilateral stimulation method, the teaching of the "internal focus of attention" and of the "external focus of attention" as cognitive anchors, the checking of the capacity to stay in the window of tolerance, and — for the paediatric protocol — the involvement of the parent.

**Phase three: the assessment.** The measurement of the subjective units of disturbance (the SUD), the body sensations, and the vividness of the image, for the selected memory, and the establishment of the baseline values that will be tracked through the processing. The clinician identifies the specific memory that will be worked on, and the choice of the memory is a clinical judgement and not a neutral matter.

**Phase four: the reprocessing.** The heart of the procedure. The young person holds the image in focus, follows the bilateral stimulation, and reports whatever comes. The clinician monitors the SUD and the body sensations, and adjusts the stimulation and the pacing, and periodically asks the young person what they are noticing.

**Phase five: the installation.** The new associations — the new images, the new beliefs, the new body sensations, the new links to the past and to the present — are explicitly installed, one at a time, with the bilateral stimulation. This phase is commonly the most useful and the most neglected, and a student should know that a practitioner who stops at the end of the reprocessing has delivered an incomplete treatment.

**Phase six: the body scan.** A bilateral stimulation period in which the young person attends to the body and reports any remaining sensations. The purpose is to complete the processing of the autonomic component.

**Phase seven: the completion.** A final bilateral stimulation period, and the young person is asked what they would like to address, and any remaining material is addressed briefly.

**Phase eight: the review.** The re-measurement of the SUD, the body sensations, and the vividness, and the review of the progress across the whole treatment.

## 3.2 The SUD and the other measures

The **subjective units of disturbance** are the young person's own rating of the distress associated with the memory, on a scale of zero to ten, or on the widely used pictorial scale, and they are the primary outcome measure within the protocol and the most important thing to chart.

The **body sensations** are the physical sensations associated with the memory, and the **vividness** is the rating of how vivid the image is. These three measures are recorded at the assessment, at the end of each reprocessing set, and at the completion, and the chart is a clinical document and should be part of the case record.

## 3.3 The bilateral stimulation methods

The methods are of three kinds, and the choice among them is made on the basis of the young person's preference, their comfort, and the evidence.

**Horizontal eye movements**, the original method, with the tracking of the therapist's hand moving from side to side.

**Bilateral auditory stimulation**, with alternating tones delivered through headphones, one ear at a time, so that the sound alternates from left to right.

**Bilateral tactile stimulation**, with the hands, or with the feet, or with the shoulders, tapped alternately.

**The butterfly variant**, in which the young person taps their own thighs alternately while tracking the therapist's moving hand with their eyes, and which is used with young children.

The most common finding in the comparative studies is that all of the effective methods work and that the choice is best made collaboratively.

# 4. The Mechanisms

## 4.1 The competing accounts

Three families of explanation have been proposed, and a student should be able to state them and the evidence for each.

**The ocular motor account.** The eye movements produce a specific effect on the autonomic nervous system, and in particular a reduction in the sympathetic arousal. The evidence for a specific autonomic effect of horizontal eye movements is weak and the findings are inconsistent, and the account does not explain why the auditory and the tactile methods work as well as they do.

**The attentional and the associational account.** The bilateral stimulation occupies the attention in a way that permits the free associative flow that the reprocessing requires, and that reduces the "avoidance" of the memory that characterises the unprocessed form. This is the account with the best support, and the support comes from the studies that found the effect with several stimulation methods and that found a reduced effect when the bilateral stimulation was replaced by a non-bilateral control.

**The "novelty" and the working-memory account.** The bilateral stimulation occupies a part of the working memory that is otherwise occupied by the avoidance of the memory, and the consequence is a displacement of the memory into a different representational system. This account has a plausible neurocognitive profile and some supportive findings, and it remains contested.

## 4.2 The consensus position

The field's current position is that the mechanism is not known, that the bilateral stimulation is a **provocative technique** rather than a treatment in its own right, and that the therapeutic work is the associative processing that the stimulation makes possible.

This position has an important clinical implication: **the practitioner is not required to believe the mechanism in order to deliver the treatment competently**, and a practitioner who is uncomfortable with the bilateral stimulation can deliver the same treatment with a different attention-holding method, and the protocol's clinical structure is what does the work.

# 5. The Paediatric Adaptations

### 5.1 The differences from the adult protocol

The paediatric protocol is a genuinely different protocol and a practitioner should know the differences.

**The assessment is longer and more play-based.** A young person of seven cannot give a detailed trauma narrative, and the clinician gathers the history through play, through the parent's account, through the drawings, and through the young person's own account in whatever form the young person can manage.

**The preparation is longer and more active.** The clinician teaches the technique explicitly, and the young person practises it, and the young person chooses the method, and the young person is taught the "safe place" and the "happy place" and the "calm place" in detail, and these are used throughout the procedure as the internal anchor.

**The parent is in the room.** The presence of a safe caregiver is the single most important element in the paediatric protocol, and the clinician actively coaches the parent during the procedure, and the parent's role is to be a secure base, to model calm, and to be available for comfort.

**The body work comes first.** The clinician spends a substantial amount of time in the preparation phase on the body's signals and on the young person's capacity to notice them, because the somatic component of the trauma memory is the last to change and the first to be triggered.

**The number of sets is small and the pace is slow.** A young person processes for a few minutes and then stops, and the sets are short, and the young person is given control over the stopping.

**The language is concrete and the images are drawn.** The young person's own drawings of the memory and of the safe place are used, and the drawings are worked with.

**The reprocessing is more affectively intense and the clinician intervenes more.** The clinician's "resourcing" interventions, in which the young person is taken back to the safe place and to the positive resource, are used frequently, and the protocol's guidance is that the clinician should not let the young person's distress exceed a certain level.

### 5.2 The evidence

The evidence for the paediatric EMDR protocol is good, and the studies are generally randomised controlled trials with standardised outcome measures, and the results are consistently positive, with large effect sizes.

The evidence is best for the **single-incident traumatic events** in childhood: the accidents, the disasters, the school shootings, the medicalprocedures, and the sexual abuse where the event is discrete. The evidence for the **developmental trauma** is much weaker, and the reason is methodological: the developmental trauma is not a discrete memory, the standard SUD measure does not apply well, and the protocol's structure assumes a specific and bounded target.

The comparative evidence shows the paediatric EMDR to be **comparable to the trauma-focused cognitive behavioural therapy** on outcome measures, and in some studies superior, and a recent network meta-analysis of the paediatric trauma treatments placed it among the first-line recommendations.

## 6. Further Reading for This Chapter

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- Bergmann, I. (2008). *EMDR: A powerful, evidence-based treatment for trauma, grief, and anxiety*. — The balanced review.
- Brown, T. A. (2013). *EMDR and the adaptive information processing model*. — The model.
- Shapiro, A. F., & Luycky, C. (2016). *EMDR and the adaptive information processing model: Potential mechanisms of change*. — The mechanism review.
- Bergmann, I. (2010). *EMDR for children: The research*. — The paediatric evidence.
- Diekstra, R. F. (2015). *Child and adolescent EMDR: A narrative review*. — The paediatric review.
- Grey, E. B. (2020). *A review of the use of EMDR in child and adolescent populations*. — The contemporary paediatric review.
- Smyth, A. (2019). *EMDR and the adaptive information processing model*. — The evidence base.
- The EMDR International Association's practice guidelines and the child and adolescent protocol. — The clinical standards.
- Kim, J. (2014). *Child and adolescent EMDR*. — The clinical adaptation.
- Chemtob, C. M. (2015). *The impact of EMDR on children and adolescents*. — The programme and school applications.
- Tibaek, S., et al. (2018). *EMDR for children and adolescents in a community setting*. — The naturalistic outcome study.


# 7. EMDR with Children and Adolescents

## 7.1 The developmental modifications in practice

The following are the modifications that a practitioner working with adolescents should know, and they are more specific than the general list in Section 5.

**The parent is the co-therapist.** The parent is in the room, and the parent's job is to be a secure base: to sit quietly, to be emotionally available, to model calm, to not ask questions about the memory, and to hold the young person's hand if the young person wants that. The clinician coaches the parent before the session and gives the parent a specific role, and the parent who has not been coached will talk, and the talking will derail the procedure.

**The safe place is a concrete sensory place, not a fantasy.** A fifteen-year-old's "safe place" is a specific and sensory description: the beach at a particular time, a specific room, a specific piece of music. The clinician elicits it in detail and the young person draws it, and the drawing is used as the anchor throughout the session.

**The "external focus of attention" is taught as a body sensation.** The young person is taught to locate a physical sensation in the body — a point on the sternum, the top of the head, the hands — and to attend to it, and the sensation is the anchor when the memory's affect rises. With young people this works better than the cognitive "external focus" that the adult protocol specifies, and the paediatric protocol says as much.

**The clinician intervenes more often.** The adult protocol is relatively permissive about the young person's distress during the reprocessing. The paediatric protocol is not: the clinician monitors the affect continuously and intervenes with a resourcing intervention when the young person's affect exceeds a manageable level, and the interventions are frequent.

**The sets are short and the young person controls the stopping.** A set is a few minutes rather than a full "round", and the young person is told explicitly at the start that they can stop at any time and that stopping is not a failure.

**The installation is done with the young person's own words.** The new associations that emerge during the reprocessing are written down, in the young person's words, and the installation goes through them one at a time, and the young person approves each one.

## 7.2 The specific applications with young people

**Single-incident accidents.** The best-evidenced application in the paediatric population, and the protocol maps almost perfectly onto the presentation, and the young person is usually stabilised and in a school and can be treated in a small number of sessions.

**School and community trauma.** The evidence from the school-based and the community-based programmes is good, and the group delivery of the first-aid version of the protocol has a substantial evidence base.

**Medical procedures.** The protocol was developed partly for young people undergoing painful procedures, and the modification for this application is the "prescripted image" technique, in which the young person is given a prepared image to hold while the painful stimulus is applied, and the practice is now well established in paediatric pain services.

**Sexual abuse.** The evidence is good for the protocol with a discrete abuse event, and the evidence is much weaker for the developmental pattern, and the practitioner must be careful about the demands the protocol makes on a young person who is still in the situation.

**Grief.** The protocol has a well-established application to the traumatic grief of sudden loss, and the paediatric grief application is described in the literature.

**Specific phobias and the anxiety disorders.** The protocol is effective, and in this population the question of whether a trauma-focused protocol is indicated at all, as opposed to the graded exposure of Chapter 25, is a live one.

## 7.3 The limits with young people

**The developmental trauma presentation is a poor fit.** The standard target of the protocol — a discrete, bounded, sensory memory — is not available in the developmental presentation, and the clinician who applies the protocol to a young person with a chronic relational trauma will either find that there is no target or will find that the target is a composite that the young person cannot hold.

**The protocol requires a capacity to stay with an affect.** A young person who dissociates in session cannot do the reprocessing, and the clinician should be prepared for the possibility that the dissociative response is a poor sign for the suitability of the protocol and not only a technical difficulty.

**The protocol's structure can pressure the young person.** The eight-phase structure, the SUD rating, the repeated questions about the memory — a young person who has been assessed repeatedly and who is being

asked to rate their distress on a scale can experience the procedure as another demand to perform.

**The parent in the room is a double-edged intervention.** The parent is the secure base, and the parent is also the person who may have been the source of the trauma, and the protocol requires the clinician to have established which before the session, and the clinical judgement required is significant.

## 8. Case Illustration: The Boy Who Kept Checking the Rear Window

**Presentation.** Ruben is twelve. Four months ago he was in a car that was hit by a van, and his mother was in the back seat with him. His mother was seriously injured and Ruben was not, physically. Since the accident, Ruben will not sit in the back of any car, he checks the rear window constantly, he has not slept in his own bed, he wets the bed twice a week, and his school attendance has dropped by half. The referral is from the paediatrician, who is asking for trauma-focused treatment.

**The history and the stabilisation.** The practitioner conducts the full history. The important findings are: no previous mental health history; no current risk; a stable and safe home; the mother is recovering well and is clinically stable; and the family are cooperative. The practitioner also takes the school and the paediatrician's details, and establishes that there is no ongoing threat.

The practitioner also establishes what the mother knows and does not know, and notes that Ruben's father, who separated from the mother before the accident and who is estranged, has not been informed. The practitioner discusses this with the mother and with the school and makes a safeguarding assessment, and the finding is that the risk to Ruben from his father is low and the question is whether informing him is in Ruben's interests, and the practitioner takes Ruben's view on this in the second session and records it.

**The preparation.** Over three sessions, the preparation is done. Ruben chooses the butterfly method, which he says he can do "without anyone noticing", and this is a good choice because it is discreet and because it gives him control. The safe place is established in detail: his grandfather's boat shed at the coast, the smell of the oil, the sound of the water, the blue chair. He draws it. The external focus is located: he and the practitioner find a point on his sternum that he can attend to, and the practitioner teaches him to move his attention there when the distress rises, and the teaching is repeated in each session.

The body work is done in the preparation phase, and it is given substantial time, because the practitioner has noticed that Ruben's first sign of distress is a change in his breathing, and the practitioner wants him to be able to notice it before the distress is visible.

The mother is coached: she sits on the boy's left, she does not speak during the processing, she breathes normally, she does not ask him what he is thinking, and if he reaches for her hand she holds it and says nothing.

**The assessment.** The chosen target is the moment of the impact, and Ruben describes it: the van came from the side, the noise, his mother's voice, and the moment after. The SUD is nine. The body sensations are in his jaw and his chest. The image is vivid. The practitioner charts all three and notes that the chart is going to be the main clinical document in this treatment.

**The reprocessing.** There are three sets over two sessions.

In the first set, the boy reports the image, the noise, his mother's voice, and then reports that the van "didn't even slow down" and that this is new information — that he had not remembered the detail that the van did

not slow down, and that it had been going fast, and that it had been going fast because of the roadworks, and that the roadworks are always there, and the practitioner notes that this is the associative processing, that the memory is being linked to the ordinary world of the road and the roadworks and the ordinary traffic.

In the second set, the boy reports the moment after the impact, and this is where the affect rises sharply, and the boy's breathing changes, and the practitioner intervenes with a resourcing intervention and takes him to the boat shed, and the boy's breathing settles, and the practitioner says: "You can come back to it, or we can stop here today." The boy says: "I want to come back."

In the third set, the boy reports something that surprises the practitioner: "I keep looking at the back window because I keep thinking Mum's going to say she's hurt again." The practitioner says nothing and lets him continue, and the boy goes on: "She's not, is she. She's at home. She's walking. I saw her walking yesterday and I didn't even see the bandage." And then he says: "I think I keep looking in case she needs me to look after her."

The practitioner recognises this as the essential material, and it is the material that the reprocessing exists to reach: the boy's checking is not a fear of a repeated accident; it is a ten-year-old's vigilance about his mother's care, and the accident has given him a reason to perform it.

**The installation.** The new associations are installed one at a time, with the bilateral stimulation. The practitioner asks the boy what he would like to remember about the accident, and the boy says: "That I looked after her, and that she was all right, and that I can stop." The practitioner installs this. The body scan is done, and the boy's remaining sensation is in his jaw, and the body scan reduces it, and the boy reports that it is "like a tooth that isn't there any more".

**The completion and the review.** The SUD is two. The body sensations are zero. The vividness is three. The boy says: "It was a bad accident. It just isn't happening now."

**The follow-up.** The practitioner asks what the boy thinks will be different at home. He says: "I think I'll sleep in my bed." The practitioner does not tell him that he will; she asks him what he would need to do, and he says: "Nothing. I think I'll just do it." The practitioner says: "Let's see."

**The outcome.** At the one-month review the boy is sleeping in his bed, has no bedwetting, has returned to school, and has not checked the rear window in the car in the last two weeks. The mother reports that he has asked to sit in the back again, and that he has chosen the music. The practitioner is clear that this is a good outcome and is equally clear that the work was the work of the reprocessing and the installation and not the reassurance.

**The interpretation.** The case illustrates the protocol and its specific pedagogic point: the target memory was the accident, and the material that mattered was the meaning attached to it, and that meaning — that the checking was a way of looking after his mother — was only accessible through the associative processing and not through the history, and would not have been reached by any of the cognitive procedures in Chapter 16. It also illustrates the paediatric discipline: the parent in the room and coached, the short sets, the resourcing intervention and the offer to stop, the chart as the document, and the installation as the phase that makes the change durable.

# 9. Evidence and Research Support

## 9.1 The status of the evidence

**The efficacy of EMDR is well established.** The method has been subject to more controlled trials than almost any other psychological treatment, and the meta-analyses report large and consistent effects for the PTSD and the phobias in adults. It is recommended as a first-line treatment in the major international guidelines.

**The paediatric evidence is good and growing.** The randomised controlled trials with children and adolescents show substantial effects on symptom measures and on the measures of functional impairment, and the effect sizes are large.

**The comparison with the trauma-focused cognitive behavioural therapy is favourable.** The network meta-analyses of the paediatric trauma treatments have placed EMDR among the first-line recommendations, generally comparable with or slightly ahead of the trauma-focused cognitive behavioural therapy, and ahead of the treatments that are not trauma-focused.

**The evidence on the mechanisms is genuinely inconclusive**, and the field's position is that the mechanism remains unknown and that the stimulation functions as a device for the associative processing. The studies that found effects with several stimulation methods, and with a non-bilateral control in some conditions, are the most informative.

**The evidence for the developmental trauma application is weak**, and the reasons are methodological and are described in Section 7.3.

### 9.2 Limitations of the evidence

**The samples are dominated by adults with single-incident trauma**, and the generalisation to children with developmental trauma is not supported.

**The measurement of the outcome within the protocol is subjective** and the SUD is a single-item measure that is vulnerable to expectancy effects, and the between-session measures are the ones on which the claims of the method have to stand.

**The fidelity measurement has been difficult**, and the protocol's eight phases and its specific instructions are detailed enough that delivery varies.

**The trials are frequently conducted by EMDR researchers**, and the allegiance effects have been examined in this literature and are substantial, and the field has itself commissioned critical reviews.

**The long-term follow-up is limited**, and the question of what happens to the young person in the years after the treatment, and whether the memory returns in a form, is under-researched.

**The comparison with the alternative trauma-focused treatments is relatively recent**, and it will take more and larger trials to establish the comparative position with confidence.

## 10. Strengths and Limitations of EMDR

### 10.1 Strengths

**It is brief.** A young person with a single-incident trauma may be treated in a small number of sessions, and brevity is a genuine clinical advantage.

**It is a first-line recommended treatment**, and the practitioner will be able to access it in the referral pathways of the major systems.

**The treatment does not require the young person to narrate the memory in detail**, which is one of its most important practical advantages in the paediatric population and one of the reasons it is well suited to young people who cannot or will not tell the story.

**It targets the autonomic component**, and the body work is integrated into the protocol rather than added to it.

**The installation phase is a specific and valuable technique** that is often omitted and that is one of the more transferable elements in the book.

**The chart is a good clinical document** and it makes the progress of the treatment visible to the young person, to the parents, and to the referrer.

### 10.2 Limitations

**The mechanism is unknown**, and a practitioner should be honest with a referring agent about this, while also being able to point to the efficacy evidence.

**The protocol's structure can pressure the young person**, and the repeated ratings and the structured questions are a poor fit with some young people.

**The developmental trauma is a poor fit**, and the protocol should not be applied to it by analogy.

**The training and the certification requirements are significant**, and the protocol should not be delivered by an untrained practitioner.

**The parent-in-the-room requirement is a genuine constraint**, and in some cases there is no safe parent, and the protocol's paediatric form then cannot be used in its standard form.

## 11. Cultural and Ethical Considerations

### 11.1 Culture, memory, and the treatment of the memory

**The meaning of a memory is culturally constituted.** The same event means different things in different contexts, and the processing of a memory is a process of linking it to a frame, and the frame the practitioner offers is culturally specific. A practitioner who assumes a universal phenomenology of traumatic memory will misdescribe the experience of some young people.

**Some events are not traumatic for the young person who experienced them**, and the decision to treat a memory as traumatic should be made on the basis of the young person's own account of it rather than on the basis of the event type. A young person who was in the accident and who says she was fine may be minimising, or may be telling the truth, and the assessment should be able to tell the difference.

**The cultural resources for processing a trauma are frequently available and are frequently ignored.** Many communities have established practices for the marking of loss and for the processing of frightening events, and the individual protocol can be respectful of these or can displace them, and the practitioner should ask.

**The language of the procedure is specific.** The protocol is translated into a great many languages, and the translation of the SUD scale, of the "safe place", and of the instructions for the bilateral stimulation is not a simple matter, and a practitioner working across languages should check the validated version.

### 11.2 Ethical considerations

**Informed consent with a developmental understanding.** The young person must consent in a form appropriate to their development, and the parent's consent and the young person's assent are both required, and the young person's ongoing consent during the procedure is monitored.

**The right to stop.** The young person must be told, at the start and repeatedly, that they can stop, and the protocol's design must make stopping easy and must not make it a failure.

**The prohibition of the compulsory retrieval of a memory.** The procedure works with the memories that the young person brings, and a practitioner who pressures a young person to retrieve a specific memory has departed from the protocol and has introduced a significant risk.

**The boundaries of the memory work.** The protocol is for the processing of a specific memory, and it is not an invitation to explore the whole history, and the practitioner who uses the reprocessing as a route into general history has left the evidence base.

**The confidentiality of the memory.** The young person must know, before the session, who will hear what is said, and the chart and the recording of the SUD are records of a young person's distress and are sensitive documents.

**The relationship to the safeguarding framework.** The disclosure of abuse within the protocol is a disclosure to a third party, and the practitioner must know the local reporting obligations, and must handle the situation with a specific plan agreed in advance.

**The prohibition on the use of the procedure with the child in a state of acute crisis.** The history and the stabilisation phases of Chapter 38 are the prerequisites, and a practitioner who proceeds to the reprocessing with an unstabled young person is doing harm.

## 12. Key Terms

**Key Terms at a Glance**

- **Eye movement desensitisation (EMD)** — The original name of Shapiro's procedure, in which a distressing memory was held in focus while the eyes moved horizontally.
- **EMDR** — Eye movement desensitisation and processing, the current name of the eight-phase protocol.
- **Adaptive information processing model** — The model in which the pathology lies in the failure to complete the processing of a memory, and the treatment in the completion of that processing.
- **Unprocessed memory** — A memory stored in a form that remains vivid, is experienced in the present tense, retains the original autonomic response, and is disconnected from the rest of the autobiographical narrative.
- **The link** — The missing connection between the unprocessed memory and the general network of autobiographical knowledge, and the element whose restoration the treatment produces.
- **Provocative technique** — The contemporary understanding of the bilateral stimulation, as a device for eliciting and sustaining the associative processing rather than as a treatment in its own right.
- **The eight phases** — History and stabilisation, preparation, assessment, reprocessing, installation, body scan, completion, and review.
- **Subjective units of disturbance (SUD)** — The young person's own rating of the distress associated with the memory, and the primary outcome measure within the protocol.
- **Body sensations** — The physical sensations associated with the memory, charted alongside the SUD and the vividness.
- **Vividness** — The rating of how vivid the image of the memory is.
- **The chart** — The clinical document recording the SUD, the body sensations, and the vividness across the phases and the sessions.
- **Bilateral stimulation** — The horizontal eye movements, the alternating auditory tones, the alternating tactile taps, and the butterfly variant.
- **The butterfly method** — The variant in which the young person taps their own thighs alternately while tracking the moving hand, used with young children and with those who need discretion.
- **Installation** — The phase in which the new associations are explicitly established, one at a time, with the bilateral stimulation; frequently the most neglected phase.
- **Body scan** — The bilateral stimulation period in which the young person attends to the body and reports any remaining sensations, completing the autonomic component.

**Key Terms at a Glance (continued)**

- **The internal and the external focus of attention** — The cognitive anchors used during the procedure, which in the paediatric protocol are taught as a body sensation.
- **The safe place** — A concrete, sensory, personally meaningful resource, elicited in detail and used as the anchor throughout the session.
- **Resourcing intervention** — The clinician's intervention that takes the young person back to the safe place and the positive resource when the affect exceeds a manageable level.
- **The set** — A short period of reprocessing, in the paediatric protocol lasting a few minutes, with the young person controlling the stopping.
- **The parent as co-therapist** — The coached role of the safe caregiver in the paediatric protocol: to sit quietly, to model calm, to be available, and not to ask questions.
- **Prescripted image** — The technique in which a prepared image is held during a painful procedure, developed from the EMDR principles in paediatric pain services.
- **The developmental trauma mismatch** — The problem of applying a protocol designed for a discrete, bounded memory to a young person whose trauma is a chronic relational pattern.
- **The single-event and the developmental distinction** — The distinction on which the strength of the paediatric evidence depends.
- **The biographical gap** — The separation of the young person from the event that is the target of the reprocessing, in which the affective component remains unprocessed while the cognitive component is accessible.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Describe the origin of the EMD procedure and the observation on which it was based.
2. Explain the controversy over the procedure and how the field resolved it.
3. State the three findings that the well-controlled studies established.
4. Explain the shift from the eye movement desensitisation to EMDR and its clinical significance.
5. State the central claim of the adaptive information processing model.
6. Explain the components of the model: the unprocessed memory, the dysfunction, the link, and the bilateral stimulation.
7. State the eight phases of the protocol and the purpose of each.
8. Explain the SUD, the body sensations, and the vividness, and the role of the chart.
9. Describe the four bilateral stimulation methods and the finding about their equivalence.
10. State the three competing accounts of the mechanism and the evidence for each.
11. State the field's consensus position on the mechanism and its clinical implication.
12. State the eight differences between the paediatric and the adult protocol.
13. Summarise the evidence for the paediatric protocol and state the two groups for which it is strong and weak.
14. State the seven specific adaptations for adolescents.
15. Explain the double-edged nature of the parent-in-the-room requirement.
16. State the four limits of the protocol with young people.
17. Describe the case of Ruben and state the two features of it that generalise.
18. Summarise the status of the evidence and its six limitations.
19. State the six strengths and the six limitations of the procedure.
20. Explain the four cultural considerations for EMDR practice.
21. State the seven ethical requirements for the procedure.

## 14. Practice Activities

**Practice and Application**

### Activity 39.1 — The Four Phases Audit (75 minutes, individual)

Take a young person from placement with a history of a frightening or frightening-adjacent event and write the eight phases as they would apply, with a specific plan for each. Then write the specific criterion you would use to decide whether to proceed to the reprocessing at all. Then write what you would do if, at the end of the preparation, you concluded that the young person was not stabilised enough. The third step is the one that the chapter is really about, and most students find it difficult to write.

### Activity 39.2 — The Installation Exercise (60 minutes, individual)

Take a case and write the installation as it would actually be spoken, in the young person's own words, with each new association given separately and with the bilateral stimulation noted between each. Then write the version that omits the installation, and compare. The comparison is usually striking, and it is the clearest demonstration in the book of why a specific technical detail matters.

### Activity 39.3 — The Mechanism Discussion (60 minutes, small group)

In groups of four, take the question "why does this work?" and produce four different answers: the ocular motor account, the attentional and associational account, the working memory account, and the "we do not know" answer. Then write the clinical decision that each answer would lead to. The last step is the important one, because it is the argument for why a practitioner can deliver the treatment competently without settling the mechanism, and the groups usually find that the clinical decisions are the same.

### Activity 39.4 — The Paediatric Suitability Screen (60 minutes, individual)

Write a suitability screen for the paediatric EMDR protocol, with a criterion for each of: the presence of a discrete, bounded target memory; the stability of the young person; the safety of the caregiver; the capacity to stay with the affect; the capacity to report the SUD; and the absence of ongoing threat. Then apply the screen to two cases from placement, one of which will fail it. The exercise teaches the selection decision, which is the decision with the largest clinical consequences in the whole protocol.

## 15. Annotated Reading Pathways

**For the protocol.** Read Shapiro (2017) for the contemporary text, and read the EMDR International Association's practice guidelines and the child and adolescent protocol. The guidelines are the clinical standard and they are short.

**For the model.** Read Brown (2013) and Shapiro and Luycky (2016) for the adaptive information processing model and the mechanism review.

**For the paediatric evidence.** Read the paediatric systematic reviews and the network meta-analysis of the paediatric trauma treatments, and read Bergmann's critical reviews. The critical reviews are essential and they are of a high quality.

**For the mechanism.** Read the comparative studies of the stimulation methods, which are the most interesting empirical literature in the field, and Stickgold's neurocognitive account.

**For the practice.** Read the paediatric protocol, and read the case literature on the use of EMDR in schools, in community settings, and in paediatric pain services, which are the applications most likely to be relevant to a practitioner in this programme.

# Practice Laboratory and Extended Worked Example

This laboratory turns **EMDR and Bilateral Stimulation** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is shapiro's protocol, the adaptive information processing model, and careful adaptation for children. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Priya is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Priya's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Priya explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Priya chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Priya has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Priya can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for EMDR and Bilateral Stimulation

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **EMDR and Bilateral Stimulation** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **EMDR and Bilateral Stimulation** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **EMDR and Bilateral Stimulation**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

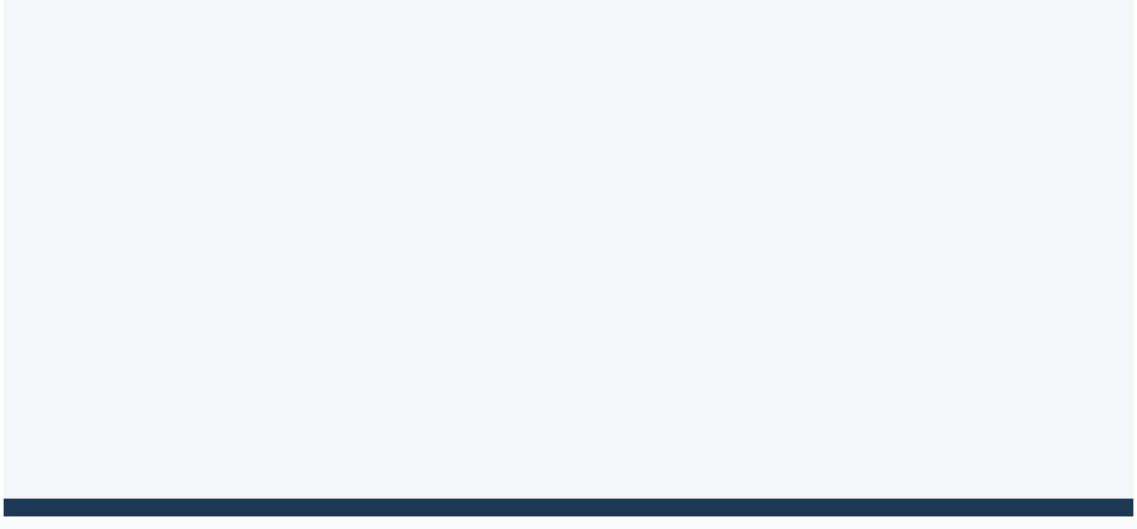
**Written consolidation.** In 250 words, explain EMDR and Bilateral Stimulation to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the

listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 40

# Stabilisation, Grounding, and Phase-Oriented Work

Skills for the first phase of trauma treatment and the maintenance of stability between sessions.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain what stabilisation is for, and distinguish it from both the processing work and the ordinary supportive conversation.
- State the core skills of a stabilisation programme and describe how each is taught to a young person.
- Describe the grounding techniques and the conditions under which each is used.
- Explain the phase-oriented approach to case management, and the criteria that determine the phase.
- Apply a stabilisation programme to a case involving a young person with severe dysregulation.
- Critically evaluate the limitations of the stabilisation work, including the risk that it becomes a way of avoiding the difficult work.

## Chapter Overview

Stabilisation is the least glamorous and the most necessary work in this book. It is the work that happens before anything else can happen: the young person who cannot remain in the room, who cannot stop checking the door, who cannot sleep, who cannot stop dissociating in the middle of a conversation, cannot do a cognitive technique, cannot process a memory, and cannot be helped by a practitioner who is trying to do either.

The central clinical proposition of this chapter is simple: **a young person's capacity to be in a relationship, to think, and to feel is a capacity that is regulated by the body, and the body can be taught to be more regulated.** A young person who is constantly in a threat state is not being difficult; the nervous system is doing what it was trained to do, and it can be trained differently, and the training takes time and repetition and it usually happens before any talk about the trauma.

The skills themselves are deceptively simple, and the chapter is largely devoted to them: the orienting to the environment, the grounding through the senses, the paced breathing, the "safe place", the "container", the "light switch", the body scan, the grounding through movement, the time travel to a safe memory, and the interpersonal skills of asking for help and of saying no. Each of them is a well-specified technique with a specific mechanism, a specific teaching procedure, and a specific set of contraindications, and a practitioner who knows them can do a great deal with a young person in the first weeks of a treatment.

The chapter also treats the two most common failures of this work: the practitioner who stabilises the young person in order to be able to do the processing work, and the practitioner who stabilises the young person instead of doing the processing work. The second is more common and is more insidious, and the chapter sets out the clinical criteria that distinguish them.

## 1. What Stabilisation Is For

### 1.1 The purpose

Stabilisation has four purposes, and a student should be able to state all four.

**To widen the window of tolerance.** A young person who is constantly hyperaroused or constantly shut down cannot process information, and the widening of the window is a precondition for everything else.

**To build the capacity to remain present.** Dissociation, avoidance, and the dorsal vagal shutdown are the young person's solutions to an intolerable state, and the solutions have to be replaced with something less costly before the original state can be addressed.

**To establish the working relationship.** A young person with a developmental trauma history will not be able to trust a practitioner until the practitioner has demonstrated that they can be relied upon to be present, to be predictable, and to be controllable — and the only demonstration that works is the demonstrated over time.

**To restore the young person's ordinary functioning.** School attendance, sleep, eating, and the relationships are the things that are immediately at stake, and the restoration of them is a legitimate goal in its own right and is frequently the thing the young person most wants.

## 1.2 What stabilisation is not

**It is not the trauma processing.** A young person can be perfectly stabilised and still have not begun to process anything, and the work must not be confused with the work.

**It is not supportive counselling.** The supportive conversation is unstructured, is about reassurance, and is of undetermined duration, and it does not build the specific capacities that the processing work requires. It is valuable and it is not the same thing.

**It is not symptom suppression.** The aim is not to make the symptoms go away; the aim is to give the young person a set of skills that they can use when the symptoms arise, so that the symptoms are no longer the only option.

**It is not a prelude that can be skipped.** A practitioner who intends to do a trauma-focused intervention and does not stabilise first will produce a worse outcome than a practitioner who never intended to do one.

## 1.3 The evidence for the stabilisation work

The evidence base here is more limited than the evidence base for the specific trauma-focused protocols, and a student should be able to describe it accurately.

**There is good evidence for the general principle** that the reduction of arousal precedes the cognitive work, that the reduction is achieved through the body, and that a young person who is dysregulated cannot use the cognitive and the narrative techniques. This principle is well supported across the psychophysiology and the clinical literature.

**The evidence for the specific individual skills is thinner and more recent.** The systematic reviews of the grounding, the stabilisation, and the self-regulation skills in the treatment of trauma report positive effects on the symptom measures and on the distress, with the most consistent findings for the paced breathing, the grounding through the senses, and the orienting.

**The evidence for the phase-oriented model as a package is thin**, and the most useful empirical support for it is indirect: the trials of the specific trauma-focused protocols with children, in which the protocols that include a substantial preparation phase do at least as well as those that do not, and the trials of the treatments that are *only* stabilisation, which show meaningful improvements in functioning and in the symptoms without any processing.

**The clinical consensus is strong** even where the evidence is thin, and this is one of the cases in the field in which the clinical consensus is better established than the empirical support, and a student should be able to

say so.

# 2. The Core Skills

## 2.1 The orienting skill

**Orient to the here and now.** The young person is asked to name, in order, five things they can see, four they can hear, three they can touch, two they can smell, and one they can taste. The technique is derived from the standard grounding exercises and it is one of the most useful in the repertoire, because it requires no training, it produces a measurable change in the autonomic state within ninety seconds, and it is available to the young person at any moment.

**Orient to the internal.** The young person is asked to name five things they can feel within their body. This is harder, it is more useful for the young person who is dissociating, and it is the bridge to the body scan.

**Orient to the temporal.** The young person is asked, in the middle of distress, to name the day, the date, the time, the season, the weather, and their age. This is a simple and effective technique for a young person who has lost track of where they are, and it is particularly useful with a young person who is dissociating or who is having a flashback.

**The teaching.** The technique is taught in a low-arousal moment, it is practised in a low-arousal moment, it is written on a card, and the young person is taught to use it proactively as well as reactively. The proactive use — using the card when the arousal is at a four rather than a nine — is the difference between a skill that is used and a skill that is not.

## 2.2 The grounding through the senses

**The five senses as an anchor.** "Feel the chair. Feel the floor under your feet. Hear the hum of the light. See one green thing. Feel the temperature of the air on the back of your hand." The technique is taught as a script and it is the most portable of the skills, because it requires nothing and it can be used in a corridor, on a bus, in a waiting room.

**The technique is for the young person who is in the room with the practitioner**, and the practitioner should use it themselves in the room, and the practitioner should say, out loud, when they are using it, because the demonstration is part of the teaching.

**The contraindication.** For a young person whose abuse involved sensory detail, the sensory grounding can be activating rather than settling, and the practitioner should ask about this before teaching it, and the alternatives are the movement-based and the orientation-based techniques.

## 2.3 The paced breathing

**The physiological rationale.** Slow exhalation lengthens the interval between heartbeats, and the vagal tone increases, and the arousal reduces. The mechanism is straightforward and it is one of the few places in this book where a specific physiological claim can be made and used with confidence.

**The technique.** The young person is taught to breathe in for a count of four and out for a count of six, or to use any other ratio in which the exhalation is longer than the inhalation. The practitioner should not over-specify: the ratio matters less than the exhalation being longer, and a young person who finds counting irritating should be given a simpler version.

**The teaching.** The technique is best taught lying down, in a low-arousal moment, with the practitioner doing it alongside the young person, and the young person is taught to notice the difference between the breathing before and after.

**The caution.** The breathing must not be used with a young person who is in the dorsal vagal state, because the deepening of the exhalation will deepen the shutdown. The breathing is for the sympathetic state; the dorsal vagal state requires orienting and movement.

## 2.4 The safe place and the container

**The safe place** is a concrete, sensory, personally meaningful location, and it is taught in detail. For a young person who finds it hard to imagine, a photograph, a place in the building, or a physical object can serve. The safe place is used as the resourcing intervention during the processing work, and it is used in the stabilisation work as a general resource.

**The container** is a different and complementary technique. The young person is asked to imagine a container — a box, a safe, a vault, a large drum — that can hold whatever they need to put away. The contents are named: the frightening images, the bad thoughts, the difficult feelings, the things that are too much. The young person then imagines placing them in the container and closing the lid. The technique is cognitively simple and it is used with young people who are ruminating or who are overwhelmed by intrusive material, and it is best taught in the drawing, with a box drawn and the contents drawn in it.

## 2.5 The body scan

**The technique.** Attention is moved through the body, from the feet to the head, with the young person reporting anything that is noticed, and nothing is interpreted. The practice is derived from the mindfulness-based body scan of Chapter 22, and the developmental adaptation for a young person with a trauma history is that the young person is in control of where the attention goes and can stop at any point.

**The two uses.** The body scan is used in the stabilisation work, as a daily practice that builds the young person's awareness of the body's signals, and it is used in the processing work, as the technique for completing the autonomic component of the memory, which is the last part to change.

**The contraindication.** The body scan is contraindicated in a young person with a history of bodily abuse, and the practitioner should ask before teaching it, and the alternative is the movement-based grounding.

## 2.6 Movement and orienting through the body

**The physical discharge.** A young person who is in a constant sympathetic state needs to move, and the movement should be large, undemanding, and not competitive: walking, running on the spot, pushing against a wall, shaking, jumping, climbing. The practitioner should be prepared to do it with the young person, and the room should be one in which this is possible.

**The orienting through movement.** The "up and down" exercise, in which the young person lies on the floor and moves their gaze up and down while a helper moves a red object, is derived from the EMDR literature and is a genuine orienting technique with a good evidence base.

**The slow, deliberate movement.** Tai chi, yoga, and structured dance have all been used in the stabilisation of young people, and the evidence base is growing, and the group format makes them practical.

## 2.7 The time travel to a safe memory

**The technique.** The young person is asked to recall a specific, real, and genuinely pleasant memory from their own life — not a fantasy, not a generic image, but a real occasion with real detail — and to place themselves in it, in the body, and to notice what they can feel now. The technique works because the recollection of a genuinely safe and pleasant experience is itself an orienting and regulating event, and because it accesses the same systems that the traumatic memory has colonised.

**The specific requirements.** The memory must be real, and it must have been checked with the young person — many young people with a difficult history have very few genuinely safe memories, and the practitioner must establish which ones they have before asking for one. The memory must be detailed. And the practice must be short, because the memory fades and the young person must not be left with the fading.

## 2.8 The interpersonal skills

Two skills are taught in the stabilisation phase because they are the skills that will keep the young person safe and are the skills that are most often missing.

**Asking for help.** The young person practises asking for help in a concrete situation, and the practice is repeated, and the response to the request is guaranteed. A young person who has never had a request met will not make one, and the point of the exercise is that the request is met, every time, in the sessions with the practitioner and in the sessions with the parents.

**Saying no.** The young person practises declining, and again the practice is repeated and the outcome is predictable. This is a skill that a young person with a trauma history frequently does not have, and it is a skill that is also a right, and a practitioner who teaches it is teaching a right as well as a skill.

## 2.9 The skill of the window of tolerance

**The traffic light.** The young person learns to rate their own arousal on a three-colour scale, and to know their own early indicators, and to know what they need at each level. At the green level, the young person is in the learning zone. At the amber level, the young person uses the grounding skills. At the red level, the young person goes to the safe place, the container, or the trusted adult, and does not attempt to process anything.

**The value of the traffic light is that it moves the responsibility for the regulation from the practitioner to the young person**, and that is the objective of the whole phase, and the practitioner who is doing the regulating for the young person has not completed the phase even if the symptoms have reduced.

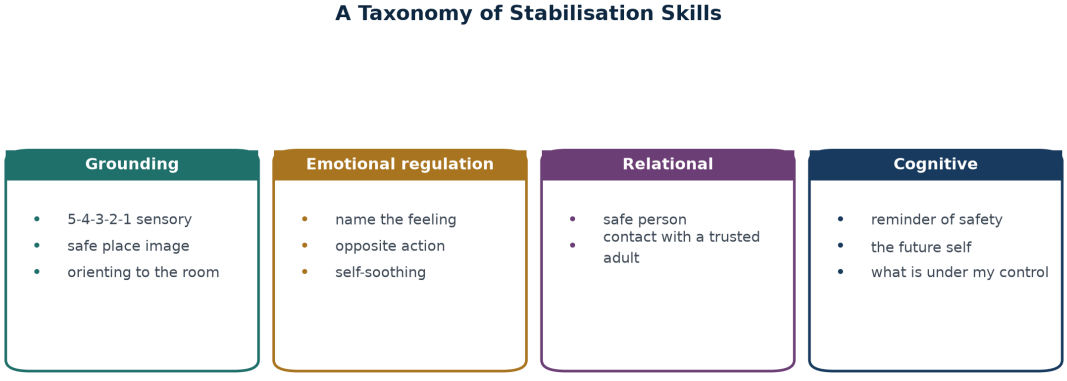



**Figure 40.28** The stabilisation skills arranged as a hierarchy, from the orienting skills through the breathing, the grounding, the body scan, the safe place, the container, and the interpersonal skills, with the traffic-light self-monitoring scheme.

## 3. Further Reading for This Chapter

- van der Kolk, B. A. (2014). *The body keeps the score*. — The neurobiological account that underwrites the body-based work.
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- Levine, H., & Frederick, A. (2012). *In an unspoken voice*. — The somatic work.
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- Boening, S., & Hülür, M. (2018). *Stabilisierung in der Traumatherapie*. — The stabilisation work in practice.
- Cutting, J. (2012). *The neuroscience of grounding*. — The evidence for the grounding techniques.
- McCann, B., et al. (2015). *Trauma and the body*. — The empirical work.
- Bryant, R. A. (2016). *Treating PTSD*. — The clinical protocol context.
- van der Kolk, B. A. (2014). The three-phase model and the clinical criteria. — The origin of the phase structure.
- International Society for Traumatic Stress Studies (2018). *Guidelines for the treatment and management of PTSD*. — The clinical standard.
- The Child and Family Traumatic Stress Intervention manual. — The preparation and stabilisation protocol.
- Linehan, M. (1993). *Cognitive-behavioral treatment of chronically parasuicidal borderline patients*. — The stabilisation skills as originally specified.
- The distress tolerance and emotion regulation skills. — The skills repertoire.
- Felitti, M. T., et al. (1998). *Relationship of childhood abuse and household dysfunction to many of the leading causes of death in adults*. — The epidemiological basis.

# 4. The Phase-Oriented Approach

## 4.1 The rationale

The phase-oriented approach is the practical response to a problem that is specific to this population. The specific trauma-focused protocols are effective and are the best-evidenced treatments available for the specific phobias and the discrete traumas of childhood, and they are not straightforwardly applicable to the developmental presentation, in which there is no discrete target, the affect is chronic rather than situational, and the young person's symptoms are pervasive rather than circumscribed.

The phase-oriented approach therefore divides the work by the state of the young person rather than by the type of trauma, and it is a management framework rather than a treatment protocol: it determines what kind of work is indicated now, and it specifies the conditions under which a different kind of work becomes indicated.

## 4.2 The phases in practice

**Phase one, the stabilisation and the safety.** The work described in Section 2, and the practical and material work: the housing, the income, the school placement, the legal status, the relationships. The phase continues until the criteria for phase two are met, and the criteria are the ones given in Chapter 38.

**Phase two, the processing.** The specific protocol, and the choice of protocol is determined by the presentation: the trauma-focused cognitive behavioural therapy or the EMDR for a discrete event; the narrative work of Chapter 36 for a young person whose difficulty is organised around a dominant story; the emotion-focused work of Chapter 37 for a young person whose difficulty is organised around an emotion; the attachment-based work of Chapter 34 for a young person whose difficulty is organised around the relationship.

**Phase three, the integration and the rehabilitation.** The rebuilding of the young person's life: the education, the relationships, the futures, the identity. The phase is the most neglected in practice and the most important for the outcome, and the developmental literature is clear that a young person whose trauma has been processed and who has not been helped to build a life is not going to stay well.

## 4.3 The phase transitions as clinical decisions

The phase transitions should be made explicitly and recorded, and the criteria are as given in Chapter 38.

**The transition into phase two is a decision about readiness, and it should be made collaboratively with the young person and, where relevant, with the caregiver.** A young person who does not want to process is not ready, whatever the criteria say, and the work at that point is the work of the phase one, and the practitioner should say so.

**The transition backwards is a decision that requires a formulation.** A young person who deteriorates in phase two is not a failure of the technique; the deterioration is information, and the usual findings are that the young person was not stabilised, or that the caregiver's behaviour has changed, or that a new crisis has occurred, or that the work has gone too fast.

**The transition into phase three is often made too early and is often not made at all**, and the most common reason is that the young person's symptoms have reduced, and the reduction of the symptoms is not the same as the restoration of the life.

### 4.4 The management framework

The phase-oriented approach is usually delivered as a **framework for the whole service** rather than as a treatment for one young person, and in this form it has a number of features that a practitioner should know.

**Phase-specific pathways.** Each phase has its own procedures, its own documentation, and its own review criteria, and the young person's record shows clearly which phase they are in.

**Phase-specific staff.** The practitioners who do the stabilisation work and the practitioners who do the processing work may be different, and this is often a good arrangement, because the demands of the two are different.

**The measurement at each phase.** The measures that are collected at each phase, and the review that compares the phase's outcomes against the criteria for the next.

**The avoidance of the false dichotomy.** The framework avoids the two failure modes: the service that does only stabilisation and never processes, and the service that processes everybody regardless of their readiness.

## 5. Stabilisation with Children and Adolescents

### 5.1 The developmental modifications

**The skills are taught to the young person, not to the practitioner.** A common error in this work is for the practitioner to be the one who does the grounding — and to be the one who does it for the young person, in the session, every time, which means the young person never acquires the skill and the practitioner never becomes unnecessary.

**The skills are taught in the language of the young person's life.** A twelve-year-old's safe place is a specific and sensory place, and a fifteen-year-old's grounding practice may be a song on a playlist, and the practitioner should ask what the young person already does to feel better and build the programme from that.

**The parent is taught the same skills.** The parent's own regulation capacity is the mediator of the young person's regulation, and the parenting work of Chapter 33 and the parental emotion coaching of Chapter 37 are the vehicles. A parent who is dysregulated cannot co-regulate a child, and the practitioner who teaches the young person the skills and does not teach the parent has done half the work.

**The skills are generalised deliberately.** A skill that is used only in the therapy room is not a skill; the practitioner should think in terms of the contexts in which the young person will need them — the corridor, the bus, the playground, the dinner table — and should plan the practice accordingly.

**The developmental order matters.** A young child is taught the sensory grounding and the safe place; an adolescent is taught the traffic light, the interpersonal skills, and the cognitive reframing of the state.

### 5.2 The presentations

**The young person with dissociation.** The dorsal vagal state is the clinical fact, and the intervention is the orienting and the physical, and the movement-based grounding, and the work on the safety, and the practitioner should not attempt to work with the content.

**The young person with hyperarousal.** The sympathetic state, and the intervention is the reduction and the containment, and the work on the sleep, on the caffeine, on the noise, on the schedule, and the physical work.

**The young person with the persistent somatic presentation.** The somatic work of Chapter 43 is the entry point, and the somatic work is frequently the stabilisation work, and a young person who has been told for years that there is nothing wrong with them will be more willing to engage with a somatic intervention than with a psychological one.

**The young person in an unstable placement.** The stability of the placement is the intervention, and the practitioner who is working on the young person's skills while the placement is in flux is working on the wrong level.

### 5.3 The limits

**Stabilisation does not resolve the trauma.** A young person who is perfectly stabilised and whose trauma is unprocessed will remain vulnerable, and a practitioner who is content with the reduction in the symptoms has substituted symptom management for treatment.

**The skills can be used to avoid.** A young person who has learned the grounding thoroughly can use it to avoid the work, and the practitioner who is pleased by the young person's mastery has been outmanoeuvred, and the question to ask is whether the young person is using the skill in the moment of distress or in the moment of the invitation to the difficult work.

**The phase one can become a waiting room.** The practitioner's anxiety about the processing work is the most common reason for the phase one continuing, and the supervisory question — "what would have to be different for us to move to phase two?" — is the most useful.

**The young person who cannot be stabilised.** Some young people cannot be, and the reasons are usually a continued threat, a caregiver who cannot provide safety, or a neurodevelopmental condition that the stabilisation programme has not been adapted for.

## 6. Case Illustration: The Young Woman Who Was Not in the Room

**Presentation.** Nadia is sixteen, and the referral is from a child and adolescent mental health service that has assessed her and diagnosed a complex post-traumatic stress disorder following a year of sexual abuse by a stepfather. The service has referred her for a trauma-focused protocol. The previous two attempts at therapy have ended: one because she did not attend after the first session, and one because the therapist's approach had made her "worse".

**The assessment.** The practitioner meets Nadia for the assessment, and the first thing they do is not to take a history. The first thing they do is to ask Nadia what happened in the previous therapy. Nadia says: "She kept asking me about it. And I'd go somewhere else. And then she'd get more upset than I had, and I'd be the one apologising." The practitioner says: "That's a specific thing. What would you have needed instead?" And Nadia says: "Somebody to ask me what I needed."

**The phase placement.** The practitioner places Nadia in phase one, and the criteria are explicit in the record: no consistent, safe adult in her life; a mother who is not able to be present with Nadia's distress because her own distress is overwhelming her; a persistent somatic presentation with a pain presentation that has not been medically explained; dissociation in the first session; and no capacity to report the SUD on a memory, which is a prerequisite of the protocol she has been referred for.

**The first three sessions.** The practitioner does not ask about the abuse in the first three sessions. The work is as follows.

*Session one.* The practitioner explains the phase model in plain language: "There are two kinds of work here. One is working out what happened and what it did to you. The other is making sure you can survive the ordinary week and sleep and be in a room with people. Most people I've worked with who had a bad experience find the second one first, because the first one needs you to be able to be in a room. So that's what we're doing first." Nadia says: "How long?" and the practitioner says: "I don't know. It depends how fast you get the skills. And we'll be checking." The practitioner then does the orienting exercise with her, in the room, and Nadia notices that she has been looking at the door the entire time, and the practitioner names this and asks about it, and Nadia says: "I'm waiting for him to come in." The practitioner says: "He's not going to come in here. I want to be clear about that. This room is yours for fifty minutes and he is not in it and he is not going to be in it."

*Session two.* The skills. Nadia chooses the grounding that she finds most tolerable, which is not the sensory one — she says the sensory one makes her think of the specifics — and it is the movement-based one and the time travel to a safe memory. Her safe memory is specific: her grandmother's kitchen, the smell of the bread, the sound of the radio. She practises it with the practitioner, and the practitioner practises with her, and Nadia's shoulders come down, and she notices it, and the noticing is the teaching.

*Session three.* The mother. The practitioner sees the mother alone, and the mother's account is that she did not know, that she is "destroyed", that she cannot sleep, that she is frightened of her own daughter because the daughter screams at her, and that she has been told by a service that Nadia "needs to process" and that she has been told by another service that she "needs to be the parent". The practitioner asks the mother the coping question — "How have you been managing?" — and the mother says: "I've been lying on the floor of the bathroom. That's where I go." The practitioner stays with that, and the mother cries, and the practitioner does not move on, and the mother says: "I've been so frightened of her. And she was the one who was hurt."

**The work with the mother.** The practitioner teaches the mother three things, and the first of them is the traffic light, which the mother learns to apply to her own arousal, and which she initially uses to describe Nadia's. The second is the orienting exercise, which the mother practises in the bathroom when she is at the point at which she would usually go and stand outside Nadia's door. The third is the specific change in the relationship: the mother stops going to Nadia's door when Nadia is in distress, and the practitioner says why: "Every time you go to that door, she learns that the way to get you is to scream, and every time she learns that, she has to scream louder, and she is spending all of her energy on it." The mother says: "But she needs me." And the practitioner says: "She needs you. She doesn't need you at the door while she's screaming. She needs you in the kitchen with a cup of tea and ten minutes of not needing anything from her."

**The assessment of the somatic presentation.** The practitioner arranges a medical review of the abdominal pain and arranges the screening for the physical consequences of the abuse, and the review finds nothing unaccounted for, and the practitioner says to Nadia: "Your body has been through something and it has kept

the record. The pain is not imaginary and it is not you being weak. What we are going to do is work with the body directly, and there is a person here who is very good at that, and I would like you to meet her." This is the referral to the somatic work of Chapter 43, and it is done in the first month, and it is the intervention that changes the trajectory of the case.

**The outcome of phase one.** At four months, Nadia is sleeping, is not dissociating in the sessions, has a safe adult in the school — the pastoral support worker, who has been trained in the grounding skills and who checks in with her daily by a specific agreed method — and the mother is no longer lying on the bathroom floor. The somatic work is running. The phase two work has not begun, and the practitioner says so to the referrer, in writing, with the criteria and the current status against each of them.

**The interpretation.** The case illustrates the central point of the chapter. The referral was for a protocol, and the protocol was the right protocol for a young person who was ready, and Nadia was not ready, and the failure of the previous therapies was a failure of the phase. The work of the first four months was not preparation in the sense of a preliminary; it was the treatment of a young woman whose nervous system was organised for survival and who had no capacity to be in a room. The mother was the decisive intervention, and the decision about the door was the specific act of clinical reasoning that made the rest possible.

# 7. Evidence and Research Support

## 7.1 The status of the evidence

**There is good evidence for the general principle** that the reduction of arousal precedes the cognitive work, and that the reduction is achieved through the body, and that a young person who is dysregulated cannot use the cognitive and the narrative techniques. This principle is well supported.

**The evidence for the individual grounding skills is growing and is positive.** The systematic reviews of the grounding, the stabilisation, and the self-regulation skills report positive effects on the symptoms of distress, on the physiological measures, and on the functional measures, and the most consistent findings are for the paced breathing, the sensory grounding, and the orienting.

**The evidence for the parent training element is the strongest part.** The parental component is the element with the best support, and the evidence for the parenting programmes in this population is good, and this is consistent with the developmental finding that the caregiver's regulation is the mediator of the young person's regulation.

**The evidence for the phase model as an integrated framework is thin**, and the most useful support is indirect: the comparative trials of the specific protocols, and the trials of the stabilisation-only treatments, which do show meaningful improvement in functioning.

**The clinical consensus considerably exceeds the empirical support**, and a student should be able to state this without embarrassment, because it is a true and important feature of the field.

## 7.2 Limitations of the evidence

**The trials are small and heterogeneous**, and the measures are frequently the self-reported distress measures.

**The fidelity of the stabilisation work is difficult to measure**, and the skill is delivered in different forms by different practitioners.

**The long-term outcomes are not well documented**, and the developmental question is unanswered.

**The studies are dominated by the adults**, and the paediatric evidence is smaller.

**The comparative evidence on the phase model against the direct provision of a specific protocol is absent**, and this is the most important missing study in the field.

# 8. Strengths and Limitations of the Stabilisation Work

## 8.1 Strengths

**It is the work that makes the rest possible.** This is its central and sufficient justification.

**It is teachable and it is portable.** A young person who has four or five grounding skills and a traffic light has something that works in the corridor, on the bus, and in the dining hall, and it does not require a therapy appointment.

**It is honest about the timeline.** The skills take weeks to months, and the young person can be told that, and this is a considerable relief to a young person who has been offered a cure in three sessions.

**It gives the young person control.** The traffic light and the self-monitoring move the regulation from the practitioner to the young person, and this is the objective of the phase.

**It addresses the ordinary functioning.** School, sleep, eating, and the relationships are the goals that the young person and the family want, and they are not the goals that the phase model privileges.

**It is the entry point for the somatic work.** The body-based work of Chapter 43 and the stabilisation work are the same work approached from two directions.

## 8.2 Limitations

**It can become a way of avoiding the work**, and this is the most serious limitation and the most common.

**It can be delivered in a way that is custodial.** A young person whose every action is monitored and whose every dysregulation triggers a response has not been given a skill; they have been given a more sophisticated supervision, and this is a real and documented failure mode in services that use the language of safety.

**The evidence for the individual skills is weaker than the practice assumes.**

**It does not address the source of the dysregulation.** If the threat continues, the skills will be used to survive it, which is what they are for, and the young person will remain dysregulated, and this is the correct behaviour and it is not a recovery.

**It can be infantilising.** The techniques were developed for adults and for young children, and the adaptation for an adolescent is a genuine design task, and a practitioner who teaches a sixteen-year-old a "safe place" in a bubble with a rabbit will lose the young person.

# 9. Cultural and Ethical Considerations

## 9.1 Culture and the body

**The body is not a universal language.** The somatic vocabulary of the stabilisation work — the grounding through the senses, the body scan, the attention to the body's signals — assumes that the body is a legitimate object of attention and that the young person has permission to attend to it. In some cultures and in some families, the body is a private and a shameful object, and the young person will not want to be asked where they feel it.

**Some of the grounding techniques are culturally inappropriate.** The "safe place" in the form of an imagined serene location is one cultural form; in some traditions, the safe place is a specific person, or a specific practice, or a specific place of worship, and a practitioner who insists on the internalised version has missed the point.

**The physiological claims are not universally applicable.** The breathing practices are drawn from a specific and largely Western scientific account, and a young person who has been taught a different breathing practice in their own tradition should be helped to adapt it rather than to abandon it.

**The movement-based grounding is the most broadly acceptable**, and it is often the entry point for a young person for whom the internal, verbal, and cognitive forms do not work.

### 9.2 Ethical considerations

**The relationship between the skills and the care.** A young person who has learned the grounding skills and who finds that the skills are used by the adults to manage them has been taught a tool for surviving the adults' interventions, and the practitioner should be alert to this.

**The risk of the custody model.** Every safety plan and every traffic light in a service is a piece of documentation, and a young person whose every action is logged has been placed in a form of soft containment, and the practitioner should ask routinely whether the documentation is a clinical tool or a control mechanism.

**The consent to the skills.** The young person should be able to decline a technique, and a young person who declines the body scan should not be pressed, and the alternatives should be offered.

**The handling of the disclosure during stabilisation.** A young person who discloses in the middle of a grounding exercise has disclosed, and the practitioner must know the reporting obligations and must not be so preoccupied with the technique that the disclosure is mishandled.

**The meaning of "safe".** A practitioner who tells a young person that the room is safe is making a promise, and a young person who has been abused in a room with an adult has good reason to doubt it, and the promise should be made carefully and kept absolutely.

**The work with the parents and the confidentiality of the young person's own experience.** The parent is coached in the same skills, and the young person should know what the parent has been taught and what the parent has been told, and the young person should have some private time with the practitioner.

**The prohibition on the use of the stabilisation phase to defer a safeguarding decision.** A young person who is still at risk requires a safeguarding response, and the stabilisation work is not a substitute for it.

## 10. Key Terms

**Key Terms at a Glance**

- **Stabilisation** — The phase of work in which the young person's capacity to be present, to think, and to feel is restored, and in which the specific capacities required for the processing work are established.
- **The four purposes of stabilisation** — To widen the window of tolerance, to build the capacity to remain present, to establish the working relationship, and to restore ordinary functioning.
- **Orienting to the here and now** — The five-four-three-two-one sensory grounding exercise, and the variants that orient to the internal sensations and to the temporal.
- **Grounding through the senses** — The script-based use of the sensory environment to anchor the young person in the present, and its contraindication where sensory detail is activating.
- **Paced breathing** — The lengthening of the exhalation to increase the vagal tone and reduce the arousal, and the technique to use in the sympathetic state.
- **The contraindication of the breathing in the dorsal vagal state** — The deepening of the exhalation deepens the shutdown, and the orienting and movement are required instead.
- **The safe place** — A concrete, sensory, personally meaningful location, taught in detail and used as the resourcing intervention.
- **The container** — The imagined receptacle into which the young person places the overwhelming material, usually taught in the drawing.
- **The body scan** — The movement of attention through the body with the reporting of what is noticed, used both in the stabilisation work and in the completion of the autonomic component of a memory.
- **The contraindication of the body scan** — Its use with a young person with a history of bodily abuse, and the movement-based alternative.
- **Movement-based grounding** — The large, undemanding physical discharge and the orienting through movement, the most broadly acceptable form of the grounding work.
- **The orienting through movement** — The up-and-down eye movement exercise with a moving object, derived from the EMDR literature.
- **Time travel to a safe memory** — The recollection of a real and specifically pleasant memory as an orienting and regulating practice, and the requirement that the memory be real and checked.
- **Asking for help and saying no** — The two interpersonal skills taught in the stabilisation phase, and the requirement that the requests be met and the declines honoured.
- **The traffic light** — The three-colour self-monitoring scheme in which the young person locates their own arousal and knows what they need at each level.

**Key Terms at a Glance (continued)**

- **The movement of the regulation to the young person** — The objective of the stabilisation phase, and the criterion by which its completion is judged.
- **The phase-oriented framework** — The management structure in which the work is determined by the young person's state rather than by the type of trauma, with the phases and the transition criteria.
- **The transition criteria** — The explicit conditions for moving between the phases, and the requirement that they be recorded.
- **The transition backwards** — The return to phase one on the deterioration of the young person's functioning, and the formulation that the deterioration requires.
- **The phase-specific pathways** — The separate procedures, documentation, and review criteria that characterise each phase in a service framework.
- **The false dichotomy** — The two failure modes of a service that does only stabilisation, and a service that processes regardless of readiness.
- **The four presentations for stabilisation** — The dissociating young person, the hyperaroused young person, the young person with a persistent somatic presentation, and the young person in an unstable placement.
- **The co-regulation principle** — The finding that the caregiver's regulation capacity mediates the young person's, and the reason the parent is taught the same skills.
- **The generalisation plan for the skills** — The deliberate practice of the skills in the contexts in which the young person will need them, and the reason the skills are portable.

## 11. Chapter Summary

Stabilisation is the necessary and unglamorous work that precedes everything else, and its central proposition is that a young person's capacity to be in a relationship, to think, and to feel is regulated by the body, and that the body can be taught to be more regulated. The chapter set out the four purposes of stabilisation — widening the window of tolerance, building the capacity to remain present, establishing the working relationship, and restoring ordinary functioning — and distinguished stabilisation from the processing work, from supportive counselling, from symptom suppression, and from a prelude that can be skipped.

The evidence was presented accurately: good support for the general principle, a growing and positive evidence base for the individual skills, the strongest support for the parent training element, thin support for the phase model as a package, and a clinical consensus that considerably exceeds the empirical support.

The core skills were described in detail with their mechanisms, their teaching procedures, and their contraindications: the orienting skills to the here and now, to the internal sensations, and to the temporal; the sensory grounding, with its contraindication where sensory detail is activating; the paced breathing, with its physiological rationale and with the crucial caution that it must not be used in the dorsal vagal state; the safe place and the container; the body scan, with its two uses and its contraindication in the case of bodily abuse; the movement-based grounding, including the orienting through movement; the time travel to a safe

memory, with the requirement that the memory be real and checked; the two interpersonal skills; and the traffic light, with the observation that its value is that it moves the responsibility for the regulation from the practitioner to the young person, which is the criterion of the phase's completion.

The phase-oriented approach was described with its rationale, its three phases in practice, the phase transitions as clinical decisions, and the management framework with its phase-specific pathways, its phase-specific staff, its measurement, and the avoidance of the false dichotomy.

The developmental modifications were described: the skills are taught to the young person and not to the practitioner, the language is the young person's own, the parent is taught the same skills, the generalisation is planned, and the developmental order matters. The four presentations were described and the four limits stated, with the risk that the skills become a way of avoiding the work identified as the most serious.

The case of Nadia demonstrated the chapter's central point. The referral was for a protocol and the protocol was appropriate for a young person who was ready, and Nadia was not ready, and the failure of the previous therapies was a failure of the phase. The work of the first four months was not preparation but the treatment of a young woman whose nervous system was organised for survival, and the decisive intervention was with the mother, and the specific decision about the door was the act of clinical reasoning that made the rest possible.

The evidence was summarised with its limits, and the strengths and limitations were set out, with the custodial failure mode and the infantilisation risk named.

The cultural analysis addressed the body as a culturally constructed object, the cultural forms of the safe place, the limits of the universality of the physiological claims, and the relative acceptability of the movement-based grounding. The ethical section addressed the relationship between the skills and the care, the risk of the custody model, the consent to the skills, the handling of a disclosure during a grounding exercise, the meaning of the promise of safety, the transparency of the parent work, and the prohibition on using the stabilisation phase to defer a safeguarding decision.

## 12. Review and Discussion Questions

**Review and Discussion Questions**

1. State the central clinical proposition of the stabilisation work.
2. State the four purposes of stabilisation.
3. Distinguish stabilisation from the processing work, from supportive counselling, from symptom suppression, and from a prelude.
4. Summarise the evidence for the stabilisation work and state the two places in which the consensus exceeds the evidence.
5. Describe the three orienting skills and their specific use.
6. Describe the sensory grounding and state its contraindication.
7. Explain the physiological rationale of the paced breathing and state its contraindication.
8. Describe the safe place and the container, and distinguish them.
9. Describe the body scan, its two uses, and its contraindication.
10. Describe the movement-based grounding and the orienting through movement.
11. Describe the time travel to a safe memory and its requirements.
12. Describe the two interpersonal skills of the stabilisation phase.
13. Describe the traffic light and explain why it is the criterion of the phase's completion.
14. Explain the rationale of the phase-oriented approach.
15. Describe the three phases in practice and the phase transitions as clinical decisions.
16. State the five features of the phase-oriented management framework and the false dichotomy it avoids.
17. State the five developmental modifications for the stabilisation work.
18. Describe the four presentations for stabilisation.
19. State the four limits of the stabilisation work.
20. Describe the case of Nadia and state the three features of it that generalise.
21. Summarise the status of the evidence and its five limitations.
22. State the six strengths and the six limitations of the stabilisation work.
23. Explain the four cultural considerations for the body-based work.
24. State the seven ethical requirements for the stabilisation phase.

## 13. Practice Activities

**Practice and Application**

### Activity 40.1 — The Personal Grounding Inventory (90 minutes, field-based)

In a placement, work with one young person to build their own grounding inventory, and record the following: their safe place, with sensory detail; their five preferred sensory anchors; their preferred movement; their best genuine safe memory; their early warning signs; their traffic light descriptions; and the two people they can ask for help. The inventory becomes a document they own, and the exercise teaches that the inventory is built from the young person's own material and not from the practitioner's.

### Activity 40.2 — The Skill Rehearsal (75 minutes, triadic)

One student plays a fifteen-year-old in a state of high arousal; one plays the practitioner; one observes. The practitioner must bring the arousal down without talking about its content, using only the body's language. The observer's task is to record every intervention that the practitioner makes and to mark the point at which the young person's state changes. The exercise teaches the most important thing in the chapter, which is that the intervention in the sympathetic state is not linguistic.

### Activity 40.3 — The Phase Criteria Audit (60 minutes, individual)

Take a case and write out the criteria for moving from phase one to phase two, marking each as met, partially met, or not met, with the evidence. Then write the three specific interventions that would move the two that are not met from "not met" to "partially met". The exercise is the discipline of the phase model, and it also reveals how easily a practitioner drifts into phase two because the pressure to do something is stronger than the discipline.

### Activity 40.4 — The Custody Audit (60 minutes, small group)

Take a real service's safety plan, traffic light, or behaviour chart and audit it from the young person's point of view. For each element, ask: does this give the young person a skill, or does it give the adults a record? Then rewrite the elements that fail the test, and identify what would have to change in the service for the rewrite to be possible. The second half of the exercise is the organisational work of Chapter 58 and it is the part that matters most.

## 14. Annotated Reading Pathways

**For the body.** Read van der Kolk (2014), Ogden's sensorimotor psychotherapy, and the trauma and the body literature. These are the theoretical basis of the body-based work and they are more convincing than the technique manuals.

**For the skills.** Read the stabilisation sections of the Child and Family Traumatic Stress Intervention manual and the Linehan skills repertoire, and read Cutting's work on the neuroscience of grounding, which is the most useful single paper on the specific techniques.

**For the phase model.** Read the phase model chapters in the standard clinical texts and the guidelines for the treatment of PTSD, and read the child and adolescent protocols of the specific approaches.

**For the parent work.** Read the parenting and co-regulation literature and the emotion coaching material from Chapter 37. The parent component is the evidence-supported core of the stabilisation work.

**For the critique.** Read the critical literature on the custodial drift of trauma services and on the over-use of safety plans. The field has been subjected to serious criticism on these grounds and the criticism has produced some of the better practice standards.

# Practice Laboratory and Extended Worked Example

This laboratory turns **Stabilisation, Grounding, and Phase-Oriented Work** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is skills for the first phase of trauma treatment and the maintenance of stability between sessions. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tunde is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tunde's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tunde explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Tunde chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tunde has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tunde can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Stabilisation, Grounding, and Phase-Oriented Work

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Stabilisation, Grounding, and Phase-Oriented Work** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Stabilisation, Grounding, and Phase-Oriented Work** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

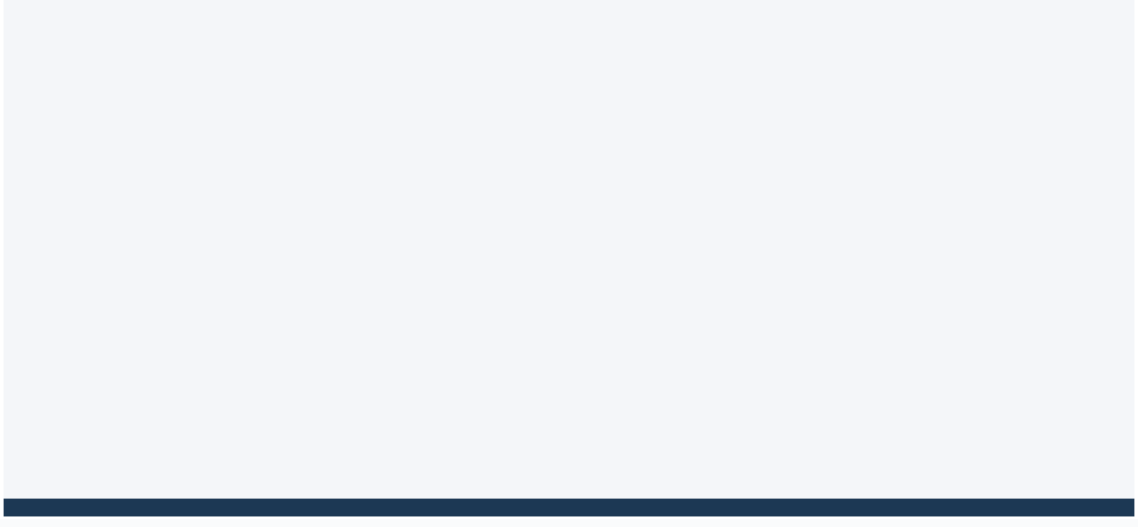
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Stabilisation, Grounding, and Phase-Oriented Work**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Stabilisation, Grounding, and Phase-Oriented Work to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 41

# Interpersonal Psychotherapy for Adolescents

Klerman's model, the social problem-solving curriculum, and the treatment of adolescent depression.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain the origin of interpersonal psychotherapy and describe the clinical problems that produced it.
- State the four problem areas of the model and describe each in detail.
- Describe the three phases of treatment and the specific tasks of each.
- Describe the techniques of the model, including the exploration, the clarification, the dialogue with the significant other, and the enactment.
- Apply the model to the treatment of adolescent depression and to a case involving an adolescent interpersonal difficulty.
- Critically evaluate the cultural and ethical dimensions of interpersonal psychotherapy with young people, including the questions of the model's origins and of the limits of a dyadic model with a young person who is at risk.

## Chapter Overview

Interpersonal psychotherapy was developed in the 1970s by Gerald Klerman and colleagues, and it was designed for a population that the psychotherapies of the time served badly: people with depression whose difficulties were not primarily cognitive, people for whom the depression had become embedded in a set of strained relationships, and people for whom a long, exploratory, and interpretative treatment was not feasible.

The model's proposition is that **depression is maintained by problems in the relationships**, and that the treatment is the resolution of those problems. The depression is not treated directly and it is not interpreted; the relationships are treated, and the depression improves as the relationships improve.

This makes it a distinctive proposition in this book, and it is worth noticing what it shares with the systemic approaches of Part V and what it shares with the behavioural approaches of Part IV. It shares with the systemic approaches the view that the problem is in the interaction rather than in the individual. It shares with the behavioural approaches the view that the targets should be specific and observable. And it shares with the cognitive approaches the view that the young person's own account of their situation is the clinical material.

The adaptation of the model for adolescents, developed in the 1990s and tested in a large multisite trial, is the most thoroughly evaluated adolescent psychotherapy in the literature for adolescent depression, and it is a treatment that a school counsellor can deliver with a modest amount of training and a good manual.

## 1. Historical Context

### 1.1 The origin

The model was developed at the Massachusetts General Hospital and its origin was partly pragmatic. The 1970s psychopharmacological trials had established that antidepressants worked, and the clinical problem was that the improvement was partial, that the relapse rate was high, and that the patients were not engaged in any long-term psychological work.

The intervention that was designed to address these problems was not a new theory of depression but a **time-limited, focused, and practical treatment** that addressed the interpersonal situations in which the patient's depression was embedded. The manual was short by the standards of the time, the treatment was twelve to sixteen weeks, and the goals were specific, current, and changeable.

### 1.2 The development of the model

The theory is described in three propositions, and a student should be able to state all three.

**Depression is a disorder of the interpersonal sphere.** The mood disorder is a response to the way in which the person's relationships are arranged, and the interpersonal difficulties are the target rather than the context.

**The role of the therapist is explicit.** The therapist is not a passive listener, not an interpreter, and not a collaborator in the exploration of the unconscious. The therapist takes an active, teaching, problem-solving role, and the relationship is warm and personal but structured, and the structure is what makes the treatment safe with a young person.

**The problems are the target and the mood improves as a consequence.** The therapist does not attempt to make the patient feel better. The therapist attempts to resolve the interpersonal problems, and the improvement in the mood is the outcome of that work, and it is a reliable outcome, and the model predicts it.

### 1.3 The evidence and the extensions

The model's evidence base is among the best in the field, and the extensions are numerous: interpersonal psychotherapy for depression, for bulimia, for social anxiety, for the PTSD, for the perinatal depression, for the grief, and for the adolescent depression.

The adolescent adaptation, which is the subject of this chapter, was developed in the 1990s and was tested in a large multisite randomised controlled trial, the Treatment for Adolescents with Depression Study, in which interpersonal psychotherapy for adolescents was one of the two treatments tested against a placebo control. The interpersonal psychotherapy arm produced significant improvements in the depression, and the improvements were comparable to those of the other active treatment, and the results are among the strongest in the adolescent depression literature.

## 2. The Four Problem Areas

### 2.1 The four areas

The model specifies four problem areas, and a student should be able to name them and describe the specific interventions associated with each.

**Grief and loss.** The death of a person to whom the young person was close, and the loss is broader than death: the loss of a relationship, the loss of a role, the loss of a place, the loss of a future. The interventions are the **complicated grief** procedure, in which the therapist works with the young person to differentiate the loss from the other things that have changed, and to restore the other things.

**Disputes and conflicts.** The relationships in which the young person is in conflict with others, most often with a parent, a sibling, or a peer. The interventions are the **dispute resolution** procedure, in which the young person is helped to identify the specific dispute, to identify the specific point of disagreement, and to

work out what specifically is being asked for and what specifically is being given.

**Deficits in role transition.** The changes of role that are developmental: leaving school, moving up, a friend moving away, a parent separating, a move to a new country, the end of a relationship, the first job. The interventions are the **role transition** procedure, in which the young person is helped to identify the roles that have been lost and the roles that are being taken on, and to explore the new role and to find the new supports.

**Deficits in social skills and interpersonal deficits.** The young person's difficulty in the relationships themselves: the difficulty in making and keeping friends, the difficulty in asserting, the difficulty in engaging with a wider network, the withdrawal. The interventions are the **social skills** and **relationship enhancement** procedures, in which the young person is helped to develop the specific skills and to practise them in role.

## 2.2 The assessment

The assessment in interpersonal psychotherapy is itself a therapeutic instrument, and the structure of the assessment is what the treatment is built on.

**The diagnostic assessment.** The diagnosis, the history, the current medication, the medical history, the current safety assessment, and the developmental and social history.

**The interpersonal inventory.** This is the distinctive instrument of the model and it is completed in a specific way: the young person is asked to name the people in their life, one at a time, and for each person to rate a set of dimensions. The dimensions are the areas in which the difficulties might lie, and the young person rates each of them on a scale, and the ratings identify the problem areas.

The dimensions are: the grieving and bereavement dimension, which asks about the loss; the disputes dimension, which asks about the conflicts; the role transition dimension, which asks about the changes; the social skills dimension, which asks about the social functioning; and the interpersonal deficits dimension, which asks about the relationships that are going badly.

**The most important person.** The inventory identifies the most important person in the young person's life, and this person becomes the focus of the treatment. In the adolescent adaptation, the most important person is usually a parent, and the treatment is therefore usually a treatment of the parent–adolescent relationship, and this is the single most important structural feature of the adolescent model.

**The specific interpersonal problem.** The interpersonal inventory ends with the identification of the specific interpersonal situation that is the current focus of the treatment, expressed as a concrete, in-the-present, and behaviourally specified request. "I want my mother to let me stay out until eleven on Fridays" is a specific interpersonal problem. "My relationship with my mum is bad" is not.

## 2.3 The role and the domains

The **role** of the young person in their own difficulty is the most clinically valuable single concept in the model, and it is worth a student's attention.

The role is asked in a specific form: "When you are with your mother and she is doing the thing that you hate, what do you do? What do you feel? What do you want?" And the three answers — the behaviour, the feeling, and the want — are then related to each other. The therapeutic work is in the relation between the three.

The clinical finding is that young people in distress frequently report a discrepancy: a role that does not match their own view of themselves, or an unsuccessful strategy, or a mismatch between what they want and what they do. The **discrepancy** is the target.

The **domains** are the specific situations in which the role and the discrepancy are observed, and the domains are agreed with the young person in the first session. In the adolescent adaptation, the domains are typically the school, the peer group, and the family, and the work moves through them in a planned sequence, usually beginning with the peer group and moving to the family.

# 3. The Three Phases

## 3.1 The structure

**Phase one: the initial phase.** Weeks one to four. The tasks are: to establish the working relationship and the contract; to provide the **education about depression**, which is a specific and empirically supported intervention in its own right; to complete the interpersonal inventory; to set the agenda for the treatment; to identify the specific interpersonal problem; to identify the domains; and to begin the work on the role and the discrepancy.

The **education about depression** deserves particular attention. It is a structured component in which the young person is told, in plain language, what depression is, what it does, what it does not mean, and what the treatment is going to do, and it has a measurable effect in its own right, and it works by normalising the experience and by removing the attribution of the depression to a personal or moral failing. The content includes: the symptoms; the prevalence; the biological and the psychological and the social contributions; the fact that it is not a character defect; the fact that it is not caused by a single event; and the fact that it is treatable. A student should be able to state this list.

**Phase two: the middle phase.** Weeks five to eleven. The tasks are the focused work on the specific interpersonal problem and on the domains, and the use of the specific techniques.

**Phase three: the termination phase.** Weeks twelve to sixteen. The tasks are: to consolidate the changes; to review the progress; to identify the remaining problems and to work on them; to review the coping skills; to make the plans for the termination; and to arrange the follow-up. The termination phase is not a summary; it is a phase with its own agenda and its own tasks.

# 4. The Techniques

## 4.1 Exploration

**Exploration** is the first of the four core techniques, and it consists of three elements.

**The exploration of the interpersonal problem.** The therapist helps the young person to specify the problem in behavioural and situational terms, and the specification proceeds by asking: who is involved; what is being said; what is being done; how often; with what consequence; and what the young person wants to be different.

**The exploration of the feelings.** The young person is helped to identify the feelings, and the specific word for the feeling is elicited, and the examination of the discrepancy between the feeling and the behaviour is begun.

**The exploration of the wishes and the wants.** The young person is asked what they want from the situation, and the question is asked separately from the feelings, and the two are then compared, and the comparison is frequently the moment of insight in the session.

## 4.2 Clarification

**Clarification** is the second technique, and it is the technical heart of the model. The therapist asks the young person to describe the interpersonal situation in specific terms, and the young person is prompted to give **details**: who was there, what was said, in what order, what the other person actually said, what the young person actually said, and what happened next.

The purpose of the clarification is to move the problem from a global and abstract description to a specific and observable event, and the effect of this is often immediate and often surprising to the young person, because a global description of a hopeless situation is transformed into a sequence of specific, and sometimes tractable, events.

The therapist's stance during the clarification is active, curious, and directive: the therapist asks, "Can you tell me exactly what your mother said?" and "And what did you say back?" and "And then what did she do?"

## 4.3 The dialogue with the significant other

The third technique is the **interpersonal dialogue**, and it is a specific behavioural procedure rather than a general encouragement to talk.

**The procedure.** The young person and the therapist construct, in the session, a script of what the young person would like to say to the significant other. The script is written down, word by word, and it is rehearsed in the session, and it is then used in the real situation between the sessions.

**The construction of the script.** The script is built from the specific problem, and the questions are: what do you want to say; what do you want them to do differently; what are you afraid will happen; and what would you say if you were not afraid.

**The rehearsal.** The rehearsal is in the session, and it is the most useful part of the technique for a young person, because it is the first time the words have been said out loud, and the young person discovers what the words sound like, and the therapist helps to revise them until they are usable.

**The practice and the follow-up.** The young person tries the script in the real situation, and the next session begins with what happened.

The clinical significance is that this technique puts the young person in the position of making a specific, specified request to a person who matters, in a form that has been rehearsed, and the effect on the relationship is often substantial, and the effect on the young person's self-efficacy is often the more important effect.

## 4.4 The enactment

The fourth technique is the **enactment**, in which the young person and the therapist stage a role-play of the problematic situation.

**The procedure.** The young person plays the significant other and the therapist plays the young person, and the scene is then reversed. The purpose is to give the young person the experience of being on the other side,

and the experience is often revelatory: a young person who believes that their mother does not listen discovers, in the enactment, that they are not listening either, and the discovery is available in a way that a direct argument would not have made it.

**The cautions.** The enactment is a technique from the psychodynamic tradition and it carries the risks of that tradition: it can be experienced as an intrusion, it can overwhelm a young person with a fragile sense of self, and it should not be used with a young person who is in the early phase of the work or who is very unstable. The therapist's competence in the technique is the precondition for its use, and the model's training in the technique is rigorous.

### 4.5 The other techniques

**The encouragement and the support.** The therapist gives direct, specific, and earned praise, and the practice of noticing and naming the young person's successes is a specific technique with specific rules.

**The symptom monitoring.** The young person monitors their own mood daily, on a simple scale, and brings the record to the session. The technique has an effect in its own right and it is a useful device for the beginning of a session, because it opens the conversation with something the young person has produced.

**The homework.** The interpersonal psychotherapy session is prepared with the agenda, and the homework is the practice of the specific technique between the sessions.

## 5. Further Reading for This Chapter

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- Thapar, A., et al. (2012). *Interpersonal therapy for adolescent depression*. — The clinical guideline evidence.

- Casey, P., & Kelly, B. (2010). *A practical guide to adolescent mental health*. — The clinical context.
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# 6. Interpersonal Psychotherapy with Children and Adolescents

## 6.1 Why the adolescent adaptation works

The adolescent adaptation works because depression in adolescence is seldom only an internal event. It is lived in bedrooms, classrooms, group chats, changing rooms, family meals, examinations, friendship ruptures, first romantic attachments, and the painful comparison between the young person's actual life and the curated lives of peers. A treatment that asks who matters, what has changed, what is disputed, and what the young person needs from the most important relationship speaks directly to the adolescent's world.

Four adaptations distinguish the adolescent model from the adult model.

**The developmental assessment is more extensive.** Puberty, school transitions, examinations, cognitive development, peer hierarchy, identity formation, digital communication, autonomy, and family dependence must all be assessed. Two young people of the same chronological age may be at very different developmental levels.

**The family is structurally involved.** The adolescent usually still lives at home, depends on adults for transport, money, permission, food, and school decisions, and cannot simply implement an interpersonal plan alone. Parents or carers are therefore brought into psychoeducation, safety planning, and selected joint sessions, while preserving a confidential individual space for the young person.

**The school is an interpersonal system.** Teachers, tutors, attendance officers, coaches, school nurses, and safeguarding leads may all hold pieces of the problem. A dispute that appears to be between mother and daughter may be maintained by a timetable, a seating plan, a public sanction system, or an unrecognised learning difficulty.

**The language is concrete.** Abstract words such as abandonment, transition, and ambivalence are translated into observable exchanges: who texted, who replied, who walked away, who raised their voice, who apologised, who was left out, and what happened next.

## 6.2 Assessment in practice

A good IPT-A assessment usually takes two or three sessions and produces four documents.

First, a **symptom and risk formulation**: diagnosis where appropriate, severity, duration, sleep, appetite, energy, concentration, anhedonia, irritability, self-harm, suicidal thinking, substance use, medical factors, medication, and protective factors.

Second, an **interpersonal inventory**: the important people, rated for closeness, conflict, reliability, support, disappointment, and change.

Third, a **problem-area formulation**: grief, dispute, role transition, or interpersonal deficit, stated in one paragraph and agreed with the young person.

Fourth, a **treatment contract**: number and frequency of sessions, who will be involved, confidentiality limits, safety arrangements, measures, and review dates.

The young person should leave assessment knowing what will happen next. Uncertainty is itself a burden for a depressed adolescent, who may already experience time as slow, effort as pointless, and help as unlikely to arrive.

## 6.3 The four problem areas with adolescents

**Grief.** Adolescent grief is often disenfranchised. The young person may be expected to return to school quickly, may not have been included in rituals, may grieve a grandparent while adults describe the death as expected, or may grieve a parent while also feeling anger, relief, guilt, or disloyalty. IPT-A distinguishes grief for a person from grief for a former self, home, school, friendship group, or imagined future.

**Role disputes.** These are the commonest presentations. A parent wants obedience and safety; the adolescent wants autonomy and trust. A teacher wants compliance; the student wants dignity and escape from humiliation. Two friends want loyalty; a third wants inclusion. The task is not to decide who is right, but to specify the non-reciprocal expectations and negotiate a tolerable next step.

**Role transitions.** Starting secondary school, moving schools, changing friendship groups, parents separating, a new sibling, migration, leaving care, examination failure, injury ending sport, pregnancy, coming out, and the transition from child to adult services are all role transitions. The old role is mourned, the new role is explored, and supports for the new role are built.

**Interpersonal deficits.** Some adolescents are chronically isolated, socially anxious, repeatedly rejected, or unable to read interpersonal cues. The work is slower and more skills-based: initiating contact, joining a group, tolerating silence, repairing after embarrassment, ending exploitative relationships, and building a small number of sustainable relationships rather than pursuing popularity.

## 6.4 Techniques translated into adolescent practice

**Communication analysis** breaks a painful exchange into a chain: trigger, interpretation, feeling, words, tone, response, consequence. A mother says, “You haven't started,” meaning concern about revision; the daughter hears, “You are lazy”; she feels shame; she replies, “Leave me alone”; the mother hears rejection; she threatens to remove the phone; the daughter withdraws; both feel unheard. The analysis makes the sequence visible without turning either person into the villain.

**Decision analysis** lists options, advantages, disadvantages, likely responses, and the young person's values. It is especially useful where every option feels intolerable.

**Role play** is used sparingly and consensually. The young person rehearses asking a teacher for an extension, telling a friend that a joke went too far, or informing a parent that constant location tracking feels like distrust. Rehearsal must never become a performance for the therapist's satisfaction.

**Affect work** links feeling to interpersonal event. The question is not merely “How depressed are you?” but “When your friend left you on read for six hours, what happened inside you, what did you conclude, and what did you do next?”

**Clarification** tests vague statements. “Everyone hates me” becomes who, when, what was said, what was inferred, and what alternative explanations exist. Unlike cognitive disputation, the goal is not to prove the thought false; it is to recover the interpersonal facts.

### 6.5 Working with parents and carers

Parents should not become co-patients by default, nor should they be excluded as obstacles. A useful structure is:

1. A parents-only psychoeducation meeting near the beginning.
2. Individual sessions with the adolescent as the main treatment space.
3. One or two planned joint sessions where a specific communication is rehearsed and attempted.
4. A termination meeting that reviews warning signs and relapse prevention.

The practitioner explains that adolescent confidentiality has limits where safety is concerned, but that ordinary disclosures about friends, sexuality, embarrassment, anger at parents, and ambivalence about treatment will not be relayed verbatim. If a parent demands a transcript, the alliance will usually collapse.

### 6.6 IPT-A in schools

Schools are natural settings for IPT-A because the interpersonal problem often occurs there and because attendance provides routine monitoring. School delivery requires clear boundaries: the counsellor is not a disciplinary officer, session content is not used for behaviour management, and outcome information shared with staff is limited to what the young person has agreed, except where safeguarding overrides confidentiality.

A school-based formulation might focus on a dispute with a teacher, grief after a friend moves away, role transition after exclusion, or social isolation following bullying. The counsellor must be alert to institutional causes. If a young person is repeatedly humiliated by public behaviour charts, private shaming, racist discipline patterns, or inaccessible work, interpersonal therapy must not be used to teach adaptation to an unjust environment. Consultation and advocacy may be required alongside therapy.

## 7. Case Illustration: The Girl Who Stopped Answering

**Presentation.** Amara is fifteen. Her grades have fallen, she sleeps until midday at weekends, and she has stopped replying to messages from her closest friend. Her mother describes her as “withdrawn and oppositional”. Her father, from whom her mother separated eight months ago, describes her as “fine when she's with me”, which increases the mother's sense of failure.

**Assessment.** Amara's most important person is her friend Sofia, not either parent. Four months earlier, Sofia began spending most breaks with a new group. Amara interpreted delayed replies as proof that she had been replaced. She stopped initiating contact to avoid further rejection, then felt guilty for being distant, then became angry when Sofia accused her of “being weird”. The primary problem area is a role dispute intertwined with a role transition: the friendship has changed, and Amara has no script for renegotiating it.

A secondary dispute exists at home. Her mother checks her phone location repeatedly after Amara stayed out late once. Amara experiences checking as surveillance; her mother experiences Amara's silence as danger. Each protects herself in a way that frightens the other.

**Formulation shared with Amara.** “Your depression worsened when the friendship changed. You protected yourself by going quiet, but the quiet made you feel more cut off and more certain that Sofia didn't care. At home, the more worried your mum becomes, the more she checks; the more she checks, the less you tell her. The treatment will focus first on the friendship dispute, then on a specific conversation with

your mum about safety and privacy." Amara agrees, but asks that her mother not be told details about Sofia.

**Middle phase.** Communication analysis reconstructs one exchange in painful detail. Sofia wrote "can't today sorry". Amara concluded, "She's done with me." She felt shame, waited three days, and replied with a joke that sounded sarcastic. Sofia replied, "why are you being like this?" Amara left the message unopened for a week.

The therapeutic work does not declare Amara's interpretation irrational. It asks what else "can't today" might mean, what Amara wanted Sofia to know, and what a direct communication would risk and make possible. Amara identifies the wish beneath the sarcasm: "I miss how it was, but I don't want to beg."

A script is drafted, shortened, and rehearsed:

> *"I know things have changed and I don't expect everything to be the same. I miss talking to you. When I go quiet, it's because I think I'm bothering you, not because I don't care. Can we just have lunch on Thursday and see how it is?"*

Amara sends a shorter version by text rather than speaking it, which the therapist accepts. Negotiating the medium is part of adolescent practice; insisting on face-to-face purity would have privileged technique over use.

Sofia replies warmly but not effusively. They meet once, awkwardly. The friendship does not return to its previous intensity, but it becomes honest. Amara grieves the old exclusivity while forming a second connection through a lunchtime art group. Depression improves not because the original relationship is restored, but because the interpersonal world becomes larger than one threatened bond.

**Family session.** A joint session addresses location checking. The therapist prevents a general trial about trust. The specific request is negotiated: location sharing during journeys to and from a Saturday activity, no continuous checking at home, a text on arrival, and review after three weeks. Amara agrees to answer a safety text within thirty minutes or explain later why she could not. Her mother agrees not to use location data to interrogate ordinary movements. The agreement is written down.

**Termination.** Amara's mood has improved, sleep is more regular, and attendance is restored. The relapse plan identifies interpersonal warning signs: not replying for more than two days, cancelling all social contact, interpreting silence catastrophically, and escalating phone conflict. She writes her own plan in her own words: "Tell one person early, even if it's just: I'm going quiet because I'm embarrassed, not because I don't care."

**Interpretation.** The case shows IPT-A's discipline. The therapist did not excavate Amara's childhood, challenge every negative thought, or instruct the mother to be less anxious. The treatment selected a focal interpersonal problem, reconstructed events in sequence, clarified feelings and wishes, rehearsed a real communication, reviewed its actual outcome, and expanded the social field. The depressive symptoms improved as the interpersonal impasse loosened.

## 8. Evidence and Research Support

Interpersonal psychotherapy has a strong adult evidence base for major depression and is recommended in several clinical guidelines. The adolescent adaptation has been evaluated in school-based and clinic-based randomised trials, including studies by Mufson and colleagues, showing greater reduction in depressive

symptoms and better social functioning than usual care or supportive comparison conditions in several samples.

The Treatment for Adolescents with Depression Study placed youth depression treatments in a large comparative context, although IPT-A was not simply interchangeable with the specific interventions tested there and should not be misdescribed. Later trials and systematic reviews support IPT-A as a credible first-line psychosocial treatment for adolescent depression, particularly where the presentation is closely linked to grief, disputes, transitions, or social deficits.

Evidence is strongest for mild to moderate adolescent depression and for delivery over approximately twelve to sixteen sessions by trained practitioners with supervision. Evidence is weaker for severe depression with acute suicidality, psychosis, bipolar disorder, severe trauma symptoms, eating disorders, substance dependence, or neurodevelopmental conditions requiring substantial adaptation. IPT-A can be part of care in such cases, but rarely the whole of care.

Outcome measurement should include depressive symptoms, functioning, school attendance, interpersonal functioning, self-harm risk, and the young person's own goals. Symptom improvement without restored participation may be insufficient; conversely, a modest symptom change with return to school, friendship, and family life may be clinically meaningful.

Limitations include variability in manual fidelity, dependence on verbal reflection, cultural assumptions about direct communication, and under-representation of some ethnic, linguistic, disabled, care-experienced, and LGBTQ+ populations in trials. Practitioners should therefore treat the model as evidence-informed, not evidence-proof.

## 9. Strengths and Limitations

### Strengths

**Developmental relevance.** Adolescent depression is often organised around belonging, autonomy, peer status, family conflict, and transitions.

**Clarity.** The focal problem area gives treatment a map and prevents aimless exploration.

**Non-blaming language.** A dispute is not a defective personality; a transition is not weakness; grief is not pathology merely because it hurts.

**Practical skills.** Communication analysis, rehearsal, and decision analysis create actions the young person can test.

**Family and school compatibility.** The model can coordinate individual, parental, and school work without collapsing confidentiality.

### Limitations

**It may understate internal processes.** Rumination, trauma memory, obsessive-compulsive mechanisms, psychosis, neurodevelopmental differences, and biological illness may require other interventions.

**Direct communication is not universally safe or culturally congruent.** In some families, direct challenge to an elder is disrespectful or dangerous. Indirect communication, mediation, written messages, or involving a trusted relative may be better.

**It can individualise institutional harm.** Bullying, racism, poverty, inaccessible schooling, and unsafe housing cannot be resolved by asking the young person to communicate more skilfully.

**It depends on a focal problem.** Diffuse distress, severe avoidance, dissociation, or inability to identify relationships may require stabilisation or another approach first.

## 10. Cultural and Ethical Considerations

Cultures differ in whether feelings are named directly, whether family decisions are individual or collective, how grief is expressed, what privacy from parents means, and how conflict with authority is handled. The practitioner must ask rather than assume: Who should be involved? What may be said directly? What would shame or endanger the young person? What forms of help are trusted?

Ethically, the practitioner must protect confidentiality while meeting safeguarding duties; include parents without surrendering the adolescent's voice; avoid becoming an agent of school discipline; and monitor risk throughout rather than assuming that a relational formulation explains everything. Consent and assent should be revisited when parents join sessions, when school staff are consulted, or when digital communication is reviewed.

Particular care is needed with LGBTQ+ adolescents. A dispute with parents may involve rejection of identity, concealment for safety, or conflict over names, pronouns, clothing, friendships, or faith-community participation. The therapist must not frame acceptance of the young person's humanity as a negotiable communication problem, nor disclose identity information without consent except where a lawful safeguarding duty requires action. Safety planning and affirming support take precedence over generic dispute resolution.

## 11. Key Terms

**Key Terms at a Glance**

- **Interpersonal psychotherapy (IPT)** — A time-limited treatment that links mood symptoms to grief, disputes, role transitions, and interpersonal deficits.
- **IPT-A** — The adolescent adaptation of interpersonal psychotherapy, incorporating developmental assessment, family involvement, and school context.
- **Interpersonal inventory** — A structured review of important relationships and the difficulties within them.
- **Role dispute** — A relationship impasse involving non-reciprocal expectations.
- **Role transition** — Distress associated with moving from an old social role to a new one.
- **Interpersonal deficit** — A persistent difficulty forming or maintaining satisfying relationships.
- **Communication analysis** — Detailed reconstruction of an interpersonal exchange to identify triggers, interpretations, feelings, responses, and consequences.
- **Decision analysis** — Structured comparison of options, consequences, values, and likely interpersonal responses.
- **Affect–event link** — The connection between a change in mood and a specific relational event.
- **Focal problem area** — The interpersonal domain selected as the primary target of treatment.

## 12. Chapter Summary

Interpersonal psychotherapy offers adolescents a focused, humane, and practical treatment that connects depression to the relationships in which it is lived. Its four problem areas—grief, disputes, transitions, and interpersonal deficits—translate complex suffering into workable questions. Who has been lost? What expectation is contested? What role has changed? What relationship capacity needs building?

The model does not ask the young person merely to think differently or behave differently. It asks what happened between people, what was meant, what was felt, what was wanted, and what could be tried next. For many adolescents, that is the exact level at which change becomes possible.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain why interpersonal psychotherapy may be especially relevant during adolescence.
2. Distinguish grief, role disputes, role transitions, and interpersonal deficits, giving one adolescent example of each.
3. What are the risks of treating a school-based interpersonal problem as though it were caused only by the student's communication style?
4. Why is confidentiality more complex when parents participate in IPT-A?
5. Compare communication analysis with cognitive restructuring. What does each method notice?
6. In the case of Amara, why was expanding her social world as important as repairing one friendship?
7. When should IPT-A be combined with, or deferred in favour of, another intervention?

## 14. Practice Activities

**Practice and Application**

### Activity 41.1 — Interpersonal inventory

Construct a fictional interpersonal inventory for a sixteen-year-old whose parent has recently remarried. Identify one likely focus area and justify excluding the others for now.

### Activity 41.2 — Communication chain

Rewrite the statement “My mum never listens” as a six-step communication chain using observable words, tone, interpretation, feeling, response, and consequence.

### Activity 41.3 — Ethical decision

A parent asks for the content of every IPT-A session as a condition of continued transport to appointments. Write a response that preserves the therapeutic frame, explains safeguarding limits, and avoids shaming the parent.

## 15. Annotated Reading Pathways

**For the model.** Read Klerman and Weissman for the adult foundations, then Mufson's adolescent adaptation. The movement from adult IPT to IPT-A shows how a treatment changes when dependence, school, family authority, and peer belonging are placed at the centre.

**For evidence.** Read the school-based IPT-A trials and systematic reviews of adolescent depression treatments. Pay attention to comparison conditions: “usual care” in a well-resourced school is not the same intervention everywhere.

**For practice.** Study communication analysis transcripts. The skill lies less in naming a problem area than in slowing an exchange down until feeling, interpretation, and wish can be distinguished.

**For cultural adaptation.** Read work on family hierarchy, indirect communication, migration, faith communities, and collectivist decision-making. Then ask how each IPT-A technique would change without losing its interpersonal focus.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Interpersonal Psychotherapy for Adolescents** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is klerman's model, the social problem-solving curriculum, and the treatment of adolescent depression. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Sofia is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Sofia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Sofia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Sofia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Sofia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Sofia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Interpersonal Psychotherapy for Adolescents

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Interpersonal Psychotherapy for Adolescents** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Interpersonal Psychotherapy for Adolescents** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Interpersonal Psychotherapy for Adolescents**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into

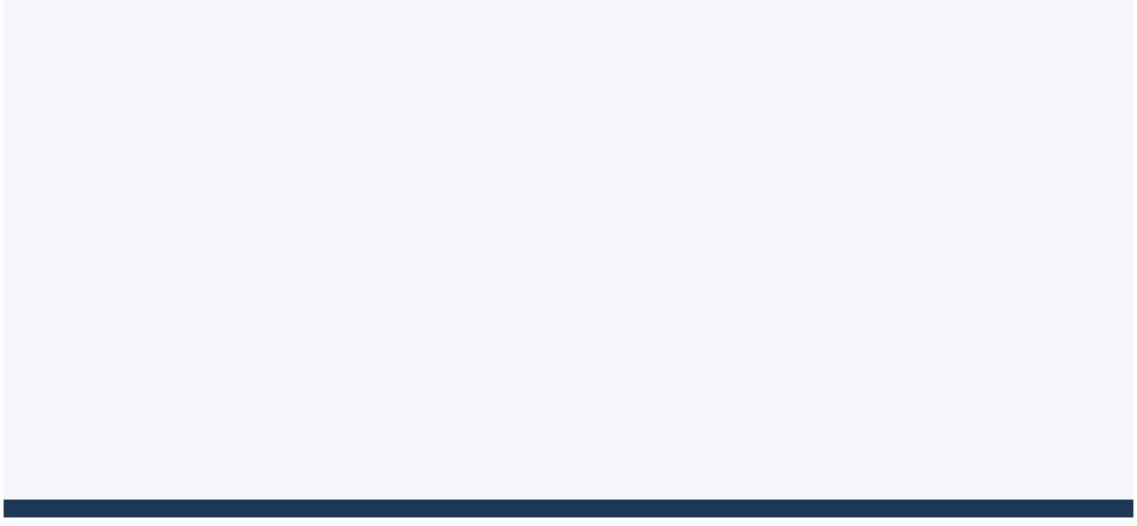
clinical governance.

**Written consolidation.** In 250 words, explain Interpersonal Psychotherapy for Adolescents to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 42

# Mentalization-Based Treatment

Fonagy's model of mentalizing, the not-knowing stance, and epistemic trust.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Define mentalizing and distinguish reflective functioning from empathy, insight, theory of mind, and emotional intelligence.
- Explain how mentalizing develops in attachment relationships and why it becomes unstable under arousal.
- Describe psychic equivalence, pretend mode, and teleological mode.
- Explain epistemic trust and its relevance to therapeutic change.
- Describe the structure and stance of mentalization-based treatment for adolescents.
- Apply mentalizing interventions to misunderstandings, conflict, self-harm urges, and family interactions.
- Evaluate evidence, limitations, cultural questions, and ethical risks.

## Chapter Overview

Mentalizing is the capacity to understand behaviour—one's own and other people's—in terms of underlying mental states: feelings, beliefs, wishes, intentions, memories, and goals. It is the ordinary human activity by which a sigh is read as disappointment rather than tiredness, silence as embarrassment rather than hostility, and one's own sudden wish to leave a room as anxiety rather than proof that everyone present is dangerous.

Peter Fonagy and colleagues argue that this capacity is not simply inborn and then expressed. It develops within early relationships in which a caregiver notices, names, mirrors, and responds to the child's internal states in a way that is marked enough to be recognisable and contingent enough to be useful. The child gradually learns: this feeling is mine; it can be thought about; other people have minds too; their behaviour can be understood, though never known with certainty.

When attachment relationships are frightening, neglectful, chaotic, or invalidating, mentalizing may develop unevenly. Under stress it can collapse. A young person who can reflect thoughtfully when calm may, after a perceived rejection, become certain that a friend hates them, that a teacher is deliberately humiliating them, or that self-harm is the only action that will prove distress to adults. Mentalization-based treatment, or MBT, aims to restore the capacity to pause, notice mental states, consider alternatives, and recover curiosity—especially at the moments when curiosity disappears.

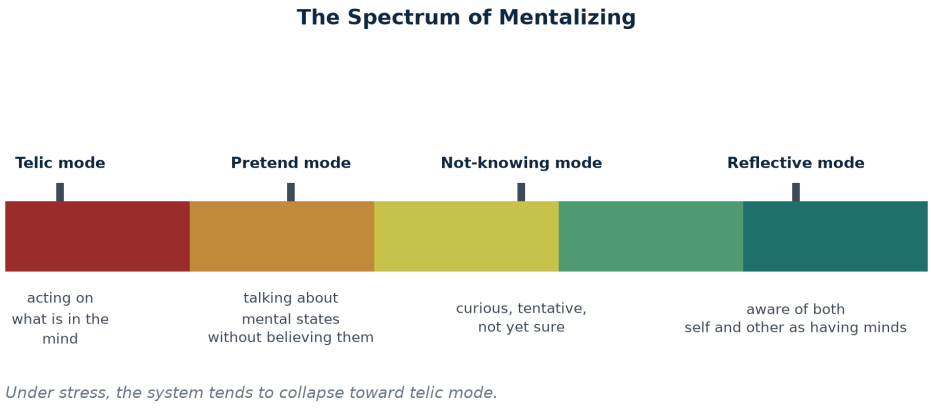



**Figure 42.29** The mentalizing spectrum, showing balanced mentalizing between automatic and controlled poles, and the collapse into psychic equivalence, pretend mode, and teleological mode under arousal.

## 1. Historical and Theoretical Context

Mentalization-based treatment emerged from psychoanalytic developmental research, attachment theory, and work with borderline personality disorder. Fonagy, Bateman, Target, and colleagues brought together several traditions: psychoanalytic interest in representation; developmental research on theory of mind; attachment research on reflective functioning; and clinical observation that people with severe personality difficulties often lose reflective capacity precisely when relationships matter most.

The model was later adapted for adolescents and families. MBT-A recognises that adolescent mentalizing is still developing. The prefrontal systems involved in controlled reflection are maturing; peer evaluation carries extraordinary salience; identity is unstable; and family relationships are being renegotiated. It would therefore be wrong to treat every adolescent failure to mentalize as pathology. Development itself involves oscillation between reflection and action, curiosity and certainty, dependence and defiance.

The clinical problem arises when collapse is frequent, prolonged, dangerous, or generalised: when misunderstandings reliably become crises, when intentions are assumed to be malicious, when feelings are experienced as facts, or when only visible action is trusted as proof of care.

## 2. What Mentalizing Is—and Is Not

Mentalizing overlaps with several familiar concepts but is not identical to any of them.

**Empathy** is primarily affective resonance and concern for another. A person may feel with someone yet misunderstand why they act.

**Theory of mind** usually refers to cognitive attribution of beliefs and intentions, often tested in developmental tasks. Mentalizing includes affect, self-experience, embodiment, and relationship context.

**Insight** implies knowledge about oneself, often verbal and retrospective. Mentalizing is a process used in real time, including when knowledge is incomplete.

**Emotional intelligence** is a broad popular and psychometric construct. Mentalizing is narrower and clinically observable: can this person, in this state, hold their own and another's perspective in mind?

**Mindfulness** involves attention to present experience. Mentalizing adds interpersonal interpretation: not only “I feel tense”, but “I feel tense because I expected criticism, and perhaps I misread her silence.”

Good mentalizing is not perfect mind reading. Indeed, excessive certainty is a failure of mentalizing. The mature position is sometimes called the **not-knowing stance**: mental states are opaque; interpretations are provisional; curiosity is preferable to premature conclusion.

# 3. Dimensions of Mentalizing

Mentalizing varies along four dimensions.

## 3.1 Automatic and controlled

Automatic mentalizing is rapid and intuitive: we sense irritation in a voice and soften our own. Controlled mentalizing is slower and reflective: we pause to ask what else could explain the behaviour. Health involves flexible movement between them. Under threat, controlled mentalizing often goes offline first.

## 3.2 Self and other

Some young people can explain everyone else's motives but have little access to their own feelings. Others are flooded by self-experience and cannot imagine the other person's position. Treatment identifies the imbalance rather than praising one side.

## 3.3 Internal and external

External mentalizing attends to facial expression, posture, tone, and behaviour. Internal mentalizing infers beliefs, wishes, and emotions. Trauma and neurodevelopmental differences may make external cues confusing; depression may make internal inference relentlessly negative.

## 3.4 Cognitive and affective

A young person may construct an intellectually elaborate account of a parent's motives while feeling nothing. Another may be overwhelmed by feeling with no narrative at all. Balanced mentalizing links thought and feeling without either dominating completely.

# 4. Attachment, Marked Mirroring, and Development

The developmental account begins with contingent mirroring. An infant expresses distress. The caregiver reflects an expression that resembles the infant's state but is marked—slightly exaggerated, softened, or framed by voice and facial cues—so that the infant can recognise it as a representation of their feeling rather than the caregiver's own overwhelming emotion.

Through repeated experiences, the child develops second-order representations: feelings become thinkable. A caregiver who is consistently frightening, absent, mocking, or overwhelmed cannot provide this reliably. The child may then experience feelings as alien, dangerous, or unrepresentable.

Adolescence offers a second developmental opportunity. Friends, teachers, mentors, romantic partners, and therapists can provide new experiences of being understood. But peer mirroring is volatile. Social media adds rapid, quantified, ambiguous feedback—likes, views, read receipts, screenshots—which can intensify mentalizing errors. A delayed reply is thin evidence, yet the adolescent mind may treat it as a verdict.

# 5. Failure Modes

## 5.1 Psychic equivalence

In psychic equivalence, internal experience is treated as external reality. “I feel rejected, therefore you rejected me.” “I am sure she is laughing at me, so she is.” There is no gap between thought and fact. Challenging the belief directly often increases defensiveness because the person experiences disagreement as denial of reality.

The intervention is not argument. It is slowing down: identifying the feeling, separating sequence from interpretation, and generating alternatives.

## 5.2 Pretend mode

In pretend mode, mental-state language becomes detached from lived reality. A young person may speak fluently about trauma, attachment, boundaries, and triggers while showing no affective connection and making no change. Therapy language itself can become pretend mode. Sessions feel sophisticated but weightless.

The intervention is to return to a concrete recent moment: What happened just before you said that? What did you notice in your body? What did you want the other person to do?

## 5.3 Teleological mode

In teleological mode, mental states are believed only when proved by visible action. “If you cared, you would answer immediately.” “If school understood, they would let me leave.” Words are discounted; only outcomes count. This mode often appears where words have repeatedly been unreliable.

The intervention validates the history behind the demand while gradually expanding what can count as evidence: consistency over time, repair after failure, and noticing intention as well as result.

## 5.4 Hypermentalizing and hypomentalizing

Hypermentalizing produces elaborate, overconfident interpretations from limited evidence. Hypomentalizing produces concrete, minimal, or absent reflection: “Dunno”, “She's just annoying”, “He did it for no reason.” Both can look like opposition. Both require pacing and curiosity rather than interpretation imposed from above.

# 6. Epistemic Trust

Epistemic trust is openness to learning from another person as a source of socially relevant knowledge. When a young person experiences a therapist as genuinely trying to understand their mind, they may become more able to consider new perspectives outside therapy. The mechanism is relational before it is

technical.

This has an important implication: MBT is not a bag of clever questions. If the practitioner is rushed, sarcastic, frightened, punitive, or mainly interested in compliance, mentalizing language will not create trust. The practitioner's own mentalizing is continuously on display, including how they handle mistakes.

When epistemic mistrust is high, the young person may treat all professional communication as manipulation. Consistency, transparency, repair, and practical reliability matter more than sophisticated formulation.

## 7. Further Reading for This Chapter

- Fonagy, P., Gergely, G., Jurist, E. L., & Target, M. (2002). *Affect Regulation, Mentalization, and the Development of the Self*.
- Bateman, A., & Fonagy, P. (2016). *Mentalization-Based Treatment for Personality Disorders: A Practical Guide*.
- Fonagy, P., & Allison, E. (2014). The role of mentalizing and epistemic trust in the therapeutic relationship. *Psychotherapy, 51*(3), 372–380.
- Rossouw, T. I., & Fonagy, P. (2012). Mentalization-based treatment for self-harm in adolescents: a randomized controlled trial. *Journal of the American Academy of Child & Adolescent Psychiatry, 51*(12), 1304–1313.
- Midgley, N., Ensink, K., Lindqvist, K., Malberg, N., & Muller, N. (2017). *Mentalization-Based Treatment for Children: A Time-Limited Approach*.
- Asen, E., & Fonagy, P. (2021). *Mentalization-Based Treatment with Families*.

## 8. The Structure of MBT-A

MBT-A usually combines individual sessions, family or parent sessions, group work in some settings, crisis planning, and close team communication. The precise format varies, but several structural principles recur.

**Assessment of mentalizing.** The practitioner observes how the young person describes self and others when calm and when aroused. Useful questions include: When a friend does not reply, what explanations occur to you? When you are angry with your mother, can you imagine what she may be frightened of? When you cut, what do you hope adults will understand?

**Formulation in mentalizing terms.** Instead of listing only symptoms, the formulation identifies triggers for collapse, characteristic failure modes, relational patterns, strengths, and recovery routes. Example: “When Mia feels ignored, she moves quickly into psychic equivalence: not being answered means being hated. She then sends escalating messages. When replies become angry, the original fear appears confirmed. She recovers mentalizing when given time, written summaries, and one consistent adult who does not retaliate.”

**A shared crisis plan.** The plan states what helps when mentalizing is lost: pause contact, move to a safe place, contact a named adult, use grounding, delay irreversible messages, and agree when communication will resume. It should be written before crisis if possible.

**Team mentalizing.** Staff also lose mentalizing under pressure. Teams begin describing a young person as manipulative, attention-seeking, or dangerous in a totalising way. MBT therefore includes team reflection

on the team's own states. This is not indulgence; it is risk management.

## 9. The Therapist's Stance

The MBT stance is active, curious, humble, and affect-focused. The practitioner frequently says, in substance:

- “I may be getting this wrong.”
- “Can we slow that moment down?”
- “What went through your mind just before you walked out?”
- “What do you imagine was happening for him?”
- “What other explanation could there be, even if it does not feel likely right now?”
- “I notice I started sounding like I was telling you off. Let me try again.”

Questions are short. Long interpretations during high arousal usually fail. The practitioner tracks shifts moment by moment: voice, posture, eye contact, certainty, humour, silence. When mentalizing collapses, the work becomes support, validation, grounding, and clarification of immediate facts. Exploration resumes when arousal falls.

The practitioner also mentalizes themselves aloud in a disciplined way. “I realise I assumed you were angry with me because you went quiet. I might have misread tiredness.” This models fallibility without making the young person responsible for the therapist's feelings.

## 10. Core Interventions

### 10.1 Stop, rewind, explore

A recent interaction is paused at the point where misunderstanding accelerated. Each step is reconstructed: event, sensation, interpretation, feeling, impulse, action, other's likely interpretation. The aim is not to assign blame but to recover sequence.

### 10.2 Mentalizing the moment

The practitioner notices a shift in the session itself. “When I looked at the clock, you went quiet. What did you make of that?” The therapy relationship becomes a live laboratory, provided the practitioner does not make every exchange about transference or pressure the young person to disclose more than is safe.

### 10.3 Affect identification and elaboration

Vague arousal is differentiated: irritation, shame, fear, loneliness, jealousy, guilt, disgust, grief. Naming does not magically regulate, but it creates a gap between impulse and action.

### 10.4 Challenge and support

MBT balances support with gentle challenge. Excess support can maintain avoidance; excessive challenge causes collapse. A useful intervention is: “I understand why sending that message felt urgent. Can we also think about what she may have read in it?”

### 10.5 Countertransference and practitioner repair

If the practitioner becomes bored, frightened, angry, rescuing, or punitive, that response is information but not instruction. Supervision helps distinguish the young person's interpersonal pull from the practitioner's history and the service's pressures. When the practitioner makes an error, repair is itself treatment.

## 11. Family Work

Families often arrive with competing certainties. A parent is certain a slammed door means contempt; the adolescent is certain a question means surveillance. Family MBT slows exchanges and asks each person to state feelings and intentions while the other listens without immediate rebuttal.

Parents are helped to distinguish behaviour from inferred motive. “He slammed the door” is observable. “He has no respect” is interpretation. The adolescent is similarly helped to distinguish a parent's anxious checking from proof of hatred or control, while the parent is still required to change behaviour that is intrusive or unsafe. Mentalizing never means the less powerful person must endlessly excuse harmful conduct.

Family sessions require ground rules: one speaker at a time, no diagnosing one another, pause when arousal rises, and a return to a concrete recent example. Where coercive control, violence, or sexual abuse is present, conjoint mentalizing work may be unsafe. Separate work and safeguarding come first.

## 12. Case Illustration: “If You Cared, You'd Answer”

**Presentation.** Lea is sixteen and has been referred after repeated self-harm following friendship conflicts. She describes intense attachments that shift quickly from idealisation to certainty of betrayal. After a friend failed to answer messages during a family meal, Lea sent twenty-seven messages, then cut her arm and sent a photograph to another friend “so somebody would actually get it”.

**Mentalizing formulation.** When calm, Lea is perceptive and funny. Perceived delay triggers abandonment fear. She enters psychic equivalence: the feeling of being ignored becomes proof of deliberate rejection. She then shifts to teleological mode: only immediate reply or visible crisis counts as care. Afterward she feels shame and concludes she is “too much”, which increases future vigilance.

**Treatment.** The therapist and Lea construct a pause plan. When she notices the urge to send a third message, she writes the unsent message in a notes application, uses cold water and paced breathing, and waits twenty minutes before deciding. She and her closest friend, with consent, discuss a realistic reply norm during school and family meals. This is not the friend becoming a clinician; it is reducing ambiguity that fuels collapse.

In session, the therapist repeatedly uses stop–rewind–explore. One exchange is slowed down second by second. Lea notices that the first thought was not “she hates me” but “I feel stupid for caring”. Shame precedes accusation. That recognition matters because the intervention changes: the task becomes tolerating shame without converting it into proof of rejection.

Her mother joins selected sessions. She learns to say, “I can see this feels urgent. I am not going anywhere, but I need us to pause before we shout,” rather than either confiscating the phone immediately or promising constant availability. Lea learns to say, “I need reassurance” before escalating.

After nine months, Lea still experiences intense jealousy and abandonment fears, but episodes are shorter, self-harm has reduced, and she can often say, “I feel dropped; I don't actually know that I am.” That sentence is not cosmetic. It marks the recovery of a gap between feeling and fact.

## 13. Evidence and Research Support

The strongest adolescent evidence is the randomised trial by Rossouw and Fonagy (2012), in which MBT-A for adolescents who self-harm produced greater reductions in self-harm and depression than usual care, with improvement in mentalizing and attachment-related measures proposed as mechanisms. Adult MBT has a substantial evidence base for borderline personality disorder, including longer-term follow-up studies.

Evidence remains less extensive than for some cognitive-behavioural treatments. Studies are relatively small; implementation requires training; comparison treatments vary; and adolescent services often adapt the model. MBT should therefore be presented as evidence-supported and promising, not as a universal answer to adolescent distress.

MBT is particularly relevant where self-harm, emotional instability, interpersonal sensitivity, attachment disruption, and repeated relational crises dominate. It is not a stand-alone treatment for psychosis, mania, severe eating disorders, substance dependence, obsessive-compulsive disorder, or acute safeguarding danger, though mentalizing principles may inform wider care.

## 14. Strengths and Limitations

Strengths include a coherent developmental theory, attention to therapeutic relationship and team states, usefulness in relational crises, and a respectful not-knowing stance. It helps practitioners understand why reasoning fails under arousal and why consistency and repair matter.

Limitations include training demands, risk of jargon, dependence on verbal reflection, and the danger of interpreting behaviour while neglecting material conditions. A hungry, unsafe, bullied, or sleep-deprived adolescent does not primarily need more sophisticated reflection. Mentalizing must sit alongside safeguarding, practical help, medical care, education advocacy, and family support.

## 15. Cultural and Ethical Considerations

Cultures differ in directness, emotional display, family hierarchy, privacy, and explanations of behaviour—including spiritual, moral, biomedical, and social explanations. The practitioner should not treat Western psychological mindedness as the sole mature form. A young person may mentalize through story, proverb, humour, silence, prayer-free communal reflection, art, music, or action rather than explicit “I think you feel” language; the task is to recognise reflective capacity in culturally available forms, without introducing religious content where the client has not chosen it.

Ethically, mentalizing must not become a sophisticated way to blame the young person for adult failures. If a parent is violent, a school discriminatory, or a placement unstable, the correct intervention may be protection and system change. Consent, assent, confidentiality, and the right not to disclose remain essential. Group work requires explicit rules about privacy and digital recording. Practitioners should avoid labelling adolescents with personality disorder language casually; developmental instability is not destiny.

## 16. Key Terms

**Key Terms at a Glance**

- **Mentalizing** — Understanding behaviour in terms of mental states such as feelings, beliefs, wishes, and intentions.
- **Reflective functioning** — The measurable capacity to reflect on mental states in oneself and others.
- **Psychic equivalence** — Treating an internal feeling or belief as external fact.
- **Pretend mode** — Mental-state talk disconnected from affect and real experience.
- **Teleological mode** — Accepting only visible action as proof of intention or care.
- **Epistemic trust** — Openness to learning from another person perceived as trustworthy and genuinely interested in one's mind.
- **Marked mirroring** — Caregiver reflection of a child's state in a form recognisable as representation rather than the caregiver's own raw emotion.
- **Not-knowing stance** — Curious acknowledgement that mental states cannot be known with certainty.

## 17. Chapter Summary

Mentalization-based treatment begins from a simple observation: when arousal rises, people lose the ability to think about minds. Adolescents may then act on feelings as facts, demand visible proof of care, or produce fluent psychological language that touches nothing. Treatment restores curiosity through a reliable relationship, slow reconstruction of interactions, affect naming, family work, crisis planning, and practitioner repair.

The model is at its best when it creates a pause—the small but decisive space in which a young person can say: this is what I feel, this is what I inferred, this is what else might be true, and this is what I will do next.

## 18. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish mentalizing from empathy and theory of mind.
2. Give adolescent examples of psychic equivalence, pretend mode, and teleological mode.
3. Why does direct argument usually fail when psychic equivalence is active?
4. Explain epistemic trust in ordinary clinical language.
5. How can a practitioner's repair of their own mistake become therapeutic?
6. What must be addressed before family mentalizing work is safe?
7. Evaluate the evidence for MBT-A and identify two limitations.

## 19. Practice Activities

**Practice and Application**

### Activity 42.1 — Stop, rewind, explore

Take a fictional two-minute argument between a parent and adolescent. Write the trigger, sensation, interpretation, feeling, impulse, action, and likely interpretation by the other person.

### Activity 42.2 — Three explanations

A friend does not reply for five hours. Generate explanations from psychic equivalence, hypermentalizing, and balanced mentalizing.

### Activity 42.3 — Practitioner repair

Write a brief repair statement for a counsellor who incorrectly assumed a quiet adolescent was angry and became defensive.

## 20. Annotated Reading Pathways

**For theory.** Fonagy and colleagues' developmental work remains foundational. Read it alongside attachment research rather than as a replacement for it.

**For practice.** Bateman and Fonagy's practical guide and Asen and Fonagy's family work show how the stance becomes interventions.

**For evidence.** Begin with Rossouw and Fonagy's adolescent self-harm trial, then examine later implementation studies and systematic reviews with attention to sample size and comparison care.

**For critique.** Read cultural and neurodiversity-informed critiques of reflective functioning measures. Ask whose communication style the instrument treats as standard.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Mentalization-Based Treatment** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is fonagy's model of mentalizing, the not-knowing stance, and epistemic trust. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Daniel is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Daniel's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Daniel explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Daniel chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Daniel has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Daniel can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Mentalization-Based Treatment

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Mentalization-Based Treatment** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Mentalization-Based Treatment** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Mentalization-Based Treatment**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

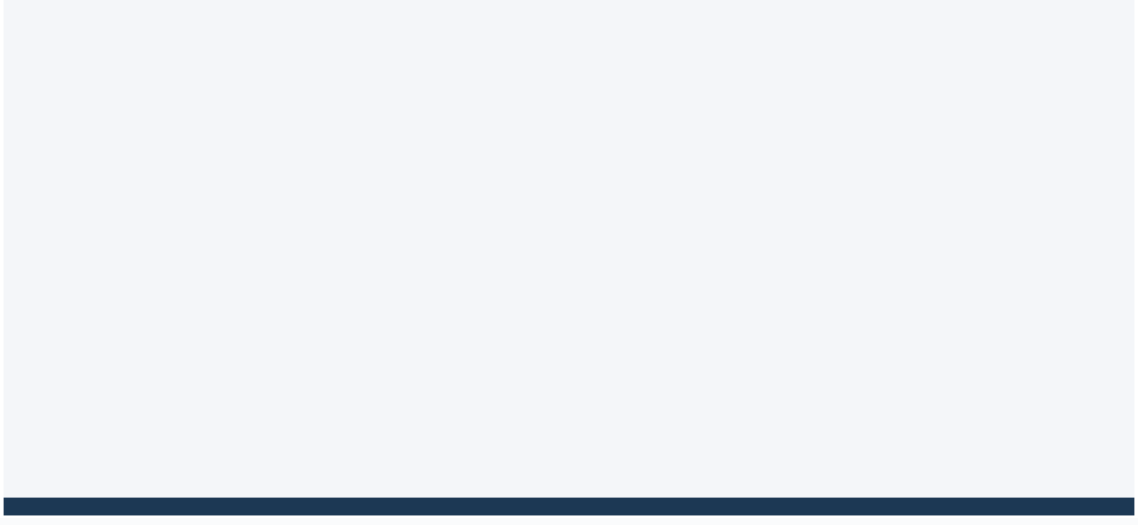
**Written consolidation.** In 250 words, explain Mentalization-Based Treatment to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the

listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 43

# Somatic, Sensorimotor, and Polyvagal-Informed Practice

Body-based approaches, autonomic regulation, and the neuroception of safety.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain why trauma, anxiety, and chronic stress may be expressed through the body.
- Describe autonomic states using polyvagal-informed language while recognising scientific debate about aspects of polyvagal theory.
- Explain the window of tolerance and identify hyperarousal, hypoarousal, and regulated engagement.
- Describe sensorimotor and somatic assessment: posture, breath, movement, sensation, impulse, and procedural patterns.
- Use grounding, orienting, breathing, movement, and boundary exercises safely.
- Distinguish somatic work from massage, physical therapy, medical treatment, and touch-based interventions requiring separate consent and competence.
- Apply somatic principles to adolescents with trauma, panic, dissociation, chronic pain presentations, and shutdown.
- Evaluate evidence, cultural considerations, safeguarding, and ethical limits.

## Chapter Overview

Some young people can describe events but not feelings. Some can recite coping strategies while their shoulders rise, breathing quickens, hands shake, or gaze becomes fixed. Others become inexplicably sleepy, blank, compliant, or absent when a subject approaches danger. Somatic and sensorimotor approaches begin with these observations: psychological life is embodied, and regulation is not achieved by instruction alone.

The word *somatic* refers to the lived body. *Sensorimotor* emphasises sensation and movement: what the body senses, prepares to do, inhibits, or repeats. Polyvagal-informed practice draws on Stephen Porges' account of autonomic regulation and social engagement, while responsible teaching must acknowledge that some specific anatomical and evolutionary claims of polyvagal theory are contested. Clinicians can use the practical language of safety, mobilisation, shutdown, and co-regulation without presenting every theoretical claim as settled neuroscience.

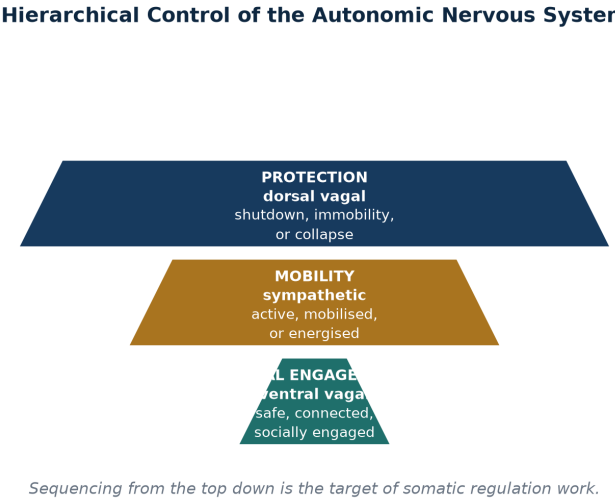



**Figure 43.30** The autonomic hierarchy, showing social engagement, mobilisation, and shutdown states, with cues of safety and danger influencing movement between them.

## 1. Why the Body Matters

Threat prepares action. Muscles tense, breathing changes, digestion slows, attention narrows, and the body prepares to fight, flee, freeze, submit, or seek help. When threat passes, a regulated system can complete the arousal cycle and return to rest, appetite, play, learning, and social connection.

When threat is chronic, unpredictable, or inescapable, adaptive responses may become habitual. A student may scan every classroom for exits, startle at a door, hold their breath when criticised, collapse into exhaustion after school, or experience stomach pain before separation. These are not character defects. They may be procedural learning: the body has learned what to do before conscious appraisal catches up.

Talk remains important, but talk delivered to a body in alarm may be heard as noise or threat. The practitioner's voice, pace, facial expression, distance, posture, and predictability are therefore clinical variables, not decorative bedside manner.

## 2. Autonomic States in Clinical Language

A simplified three-state map is useful for teaching.

**Social engagement and relative safety.** Breathing is flexible; voice has prosody; facial muscles move; eye contact can be chosen; digestion and rest are possible; curiosity returns. The young person can listen, disagree, joke, and learn.

**Mobilisation.** Sympathetic arousal supports fight or flight. The young person may be restless, irritable, argumentative, panicky, vigilant, unable to sit still, or desperate to leave. The clinical need is not immediate insight but reduction of danger cues and safe discharge or settling of energy.

**Shutdown or immobilisation.** The young person may become quiet, numb, slowed, blank, dissociated, overly compliant, or unable to speak. Adults often misread this as calm or defiance. The clinical need is

gentle orientation, choice, movement, sensory input, and connection without demand.

These states are not moral ranks and not diagnoses. A person moves among them. Shutdown may be life-preserving during unavoidable threat. Mobilisation may be appropriate in sport or emergency. Problems arise when transitions are rigid, prolonged, or triggered by cues that no longer signal actual danger.

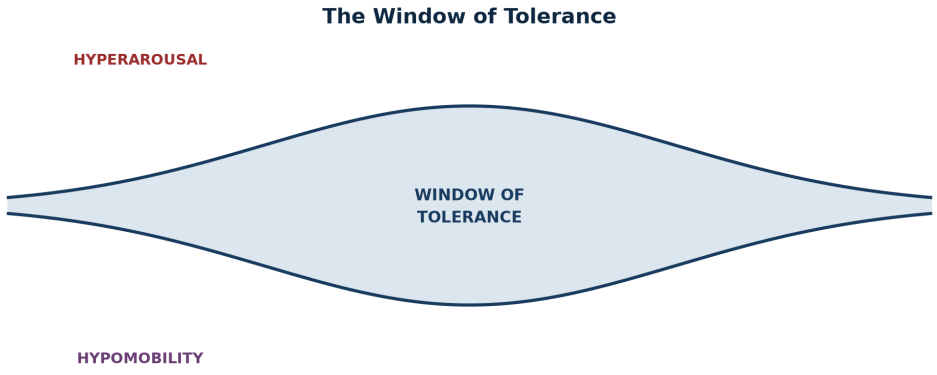



**Figure 43.31** The window of tolerance, showing regulated arousal between hyperarousal and hypoarousal, and the widening of the window through safety, skills, and co-regulation.

## 3. The Window of Tolerance

Daniel Siegel popularised the window of tolerance as the arousal band within which a person can process emotion and information without becoming overwhelmed or shutting down. Trauma, sleep deprivation, pain, hunger, illness, sensory overload, and ongoing danger narrow the window. Safety, rhythm, predictability, relationships, movement, rest, and skills widen it.

Assessment therefore asks: What narrows this young person's window? What are the first signs of leaving it? What helps return? Who helps? Which environments make regulation easier? What does the young person already do—music, walking, drawing, sport, humour, prayer-free private reflection, being with animals, coding, cooking—that genuinely settles the body rather than merely distracting adults?

## 4. Sensorimotor Assessment

Assessment is observational and collaborative. The practitioner notices, without over-interpreting:

- posture: collapsed, braced, twisted away, frozen, restless;
- breath: shallow, held, rapid, sighing, mainly upper chest;
- movement: fidgeting, rocking, leg shaking, immobility, repeated gestures;
- voice: monotone, rushed, whispering, unusually loud;
- gaze and facial expression;
- reported sensation: pressure, heat, nausea, tingling, pain, emptiness;
- action impulses: urge to run, hide, hit, curl up, shout, cry, laugh, or disappear.

Questions are concrete: “As you mention the classroom, what changes in your body?” “Where does the tightness begin?” “Does your body want to move closer, back away, or become very still?” The young person may answer with metaphor—“a knot”, “static”, “a wall”—and the metaphor should be respected rather than translated too quickly.

Medical assessment matters. Pain, thyroid disease, anaemia, asthma, cardiac symptoms, medication effects, substance use, seizures, sleep apnoea, and many other conditions can mimic or amplify anxiety and dissociation. Somatic practice never replaces medical evaluation.

## 5. Principles of Intervention

**Safety and choice.** The young person controls pace, may keep eyes open, may decline an exercise, and knows they can stop. Choice is regulating.

**Titration.** Work occurs in small doses. Attention moves between a difficult sensation and a resource—a stable chair, a pleasant image, a steady breath—rather than flooding the system.

**Pendulation.** The practitioner helps the young person move attention between activation and settling, building confidence that states can change.

**Completion and discharge.** After mobilisation, the body may need walking, shaking out limbs, stretching, drinking water, or rest. The goal is not dramatic catharsis.

**Co-regulation before self-regulation.** A calm adult, predictable routine, and relational repair precede expectations that the young person regulate alone.

**Meaning follows regulation.** Narrative linking—“this stomach tightening happens when you expect criticism”—is more useful after the sensation can be noticed without overwhelm.

## 6. Further Reading for This Chapter

- Porges, S. W. (2011). *The Polyvagal Theory*.
- Dana, D. (2018). *The Polyvagal Theory in Therapy*.
- Ogden, P., Minton, K., & Pain, C. (2006). *Trauma and the Body: A Sensorimotor Approach to Psychotherapy*.
- Ogden, P., & Fisher, J. (2015). *Sensorimotor Psychotherapy: Interventions for Trauma and Attachment*.
- van der Kolk, B. (2014). *The Body Keeps the Score*.
- Levine, P. A. (2010). *In an Unspoken Voice*.
- Siegel, D. J. (2012). *The Developing Mind* (2nd ed.).

## 7. Core Practices

### 7.1 Orienting

Orienting uses sight, hearing, and proprioception to establish present safety. The young person slowly looks around, names colours or objects, notices support from the chair and floor, and identifies exits without rehearsing escape. The exercise must be adapted where the current environment is genuinely unsafe;

pretending safety is not treatment.

### 7.2 Grounding

Grounding anchors attention in present sensory information: feet on the floor, back against a chair, holding a textured object, listening for near and far sounds, splashing cold water, or naming five observable details. Grounding is not a command to “calm down”. It is an invitation to collect evidence of here and now.

### 7.3 Breath

Slow breathing with a comfortable extended exhale can reduce arousal for some people. Breath work can also provoke panic, traumatic memories, dizziness, or a sense of control. Alternatives include breathing through movement, humming, singing, walking rhythm, or simply noticing natural breath. No adolescent should be forced to close their eyes or breathe in a prescribed pattern.

### 7.4 Movement and posture

Small experiments are preferable to dramatic release. Press feet into the floor. Push palms together. Stand and feel balance. Uncurl slowly from a collapsed posture. Walk the corridor. Shake hands and shoulders. Notice whether energy, breath, or emotion changes. The practitioner asks, “What is different?” rather than announcing what should be felt.

### 7.5 Boundaries in the body

Boundary work may involve noticing comfortable distance, practising saying “stop”, imagining a protective space, choosing where to sit, or using gesture to mark “enough”. For adolescents whose boundaries have been violated, bodily choice is therapeutic content, not a preliminary courtesy.

### 7.6 Tracking sensation and impulse

The practitioner helps sequence experience: sensation, emotion, impulse, thought, action. “My chest tightens; I feel trapped; I want to run; I think she'll shout; I leave.” Once sequence is visible, intervention points multiply. The young person might move, ask for a pause, clarify the other's intention, or choose a safer exit—not merely suppress leaving.

## 8. Somatic Work with Adolescents

Adolescents may find body-focused work embarrassing, especially amid puberty, body image concerns, disability, chronic illness, gender dysphoria, cultural modesty, or previous medical trauma. Language should be neutral and permission-based. Mirrors, touch, close physical proximity, and comments about appearance are avoided unless clinically essential, separately justified, consented, and within professional scope.

Sport, dance, martial arts, drama, music, outdoor activity, and occupational therapy may provide regulating experiences, but adults should not label every enjoyable activity “therapy” or remove its intrinsic value by over-clinicalising it. Sometimes the best somatic intervention is helping a young person safely return to football, swimming, drumming, or walking a dog.

Sleep is a somatic intervention. So are regular meals, treatment of pain, reduced late-night caffeine and energy drinks, management of sensory overload, and a school timetable that does not demand constant vigilance. A counsellor who ignores these basics while teaching sophisticated nervous-system language has reversed the clinical order.

## 9. Case Illustration: The Student Who Went Blank

**Presentation.** Tomas, fourteen, is repeatedly sanctioned for ignoring teachers. During conflict he stares, stops answering, and later cannot recall instructions. Staff interpret this as selective defiance. At home he becomes loud and restless after school, then sleeps for hours.

**Assessment.** The counsellor notices that when a male teacher raises his voice in a role-played example, Tomas' shoulders rise, breathing stops briefly, gaze fixes, and his hands become still. Tomas reports “everything goes fuzzy”. He denies choosing not to answer: “I can hear, but words don't come.” There is a history of witnessing severe domestic conflict before age nine. Medical screening and hearing assessment are arranged to avoid assuming a trauma explanation.

**Formulation.** Tomas oscillates between mobilisation and shutdown. Raised voices and public correction rapidly narrow his window. Stillness is a protective hypoarousal response, not evidence of calm. After school, accumulated arousal discharges through restlessness, followed by exhaustion.

**Intervention.** Work begins without trauma narration. Tomas maps early signs: ears “buzz”, jaw tightens, thoughts slow. He chooses signals with teachers: a break card, a seat near the door, and permission to stand at the back for two minutes rather than leaving unsupervised. Staff are coached—through proper consent and school consultation—to lower voice volume, give one instruction at a time, and check understanding after he has settled.

In counselling, Tomas practises orienting with eyes open, pressing feet into the floor, slow walking, and extending exhale only when it feels comfortable. He dislikes body scans because attention to his chest increases panic; that preference is respected. He learns to say, “I need a minute; I'm not refusing.” Teachers learn to hear that sentence as the beginning of regulation, not a challenge.

His mother is helped to create an after-school decompression routine: food, thirty minutes without questioning, physical activity, then homework in two short blocks. The counsellor does not ask Tomas to recount violent incidents. Later, when stability improves, he chooses to discuss one memory in a trauma-focused treatment with a trained clinician.

**Outcome.** Sanctions decrease. Tomas still becomes blank occasionally, but he recognises early signs and uses the break card before shutdown. The central change is interpretive as well as physiological: adults stop treating stillness as proof of contempt, and Tomas stops treating his body as evidence that he is “broken”.

## 10. Evidence and Critical Evaluation

Evidence for body-oriented trauma therapies is promising but uneven. Sensorimotor psychotherapy, somatic experiencing, trauma-sensitive yoga, breath and movement interventions, and related approaches have supportive studies, but many trials are small, heterogeneous, and difficult to blind. Outcomes may reflect therapeutic relationship, expectancy, exercise, sleep improvement, and general support as well as specific mechanisms.

Polyvagal theory has been clinically influential, especially in translating safety and co-regulation into practice. However, scholars dispute aspects of its evolutionary sequence, vagal measurement, and interpretation of respiratory sinus arrhythmia. Ethical teaching therefore distinguishes between: well-supported observations that safety cues and relationships affect arousal; plausible clinical heuristics about states; and contested neuroanatomical claims. Students should be suspicious of any programme that claims a proprietary exercise can diagnose trauma from heart-rate patterns or “reset” a nervous system in one session.

## 11. Ethics, Touch, and Safeguarding

Touch requires exceptional care with minors. Default practice in counselling should be no therapeutic touch unless there is a clear rationale, professional training, organisational policy, informed parental consent where required, the young person's ongoing assent, a chaperone or visibility where appropriate, and immediate respect for withdrawal. Many somatic interventions require no touch at all.

The practitioner must avoid suggesting that bodily responses prove a particular history. A startle response does not prove abuse; pelvic pain does not prove sexual trauma; dissociation has multiple causes. Somatic work must not become a memory-recovery technique. If disclosures occur, safeguarding procedures govern the response.

Cultural practices around modesty, gender, disability, movement, eye contact, and bodily display vary. Ask what is acceptable. Offer alternatives. Never require removing clothing, being filmed, closing eyes, or physical contact as proof of engagement.

## 12. Key Terms

**Key Terms at a Glance**

- **Somatic** — Relating to lived bodily experience.
- **Sensorimotor** — Linking sensation, posture, movement, and action impulses.
- **Co-regulation** — Regulation supported through a safe relationship.
- **Titration** — Working with activation in small, manageable doses.
- **Pendulation** — Moving attention between distress and resource to build flexibility.
- **Window of tolerance** — The arousal range in which processing and learning remain possible.
- **Orienting** — Using sensory attention to establish present-time safety.
- **Procedural learning** — Bodily patterns learned through repetition rather than conscious declaration.

## 13. Chapter Summary

Somatic and sensorimotor practice teaches counsellors to notice the body without worshipping or over-interpreting it. Breath, posture, sensation, movement, sleep, pain, and relational safety provide

information and points of intervention. Used humbly, these approaches help young people recognise state changes, widen their window of tolerance, and recover choice before, during, and after talk-based work.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish mobilisation, social engagement, and shutdown in observable terms.
2. Why can a quiet adolescent be more dysregulated than a noisy one?
3. Give three alternatives when breath-focused work increases panic.
4. Why is choice itself a regulating intervention?
5. Which claims of polyvagal-informed practice should be taught cautiously?
6. Write a no-touch somatic plan for a student who dissociates during conflict.
7. When should medical assessment take priority over somatic formulation?

## 15. Practice Activities

**Practice and Application**

### Activity 43.1 — State map

Create a personal state map: early bodily signs, thoughts, impulses, helpful people, places, movements, and sensory resources for hyperarousal and hypoarousal.

### Activity 43.2 — Micro-experiment

Practise orienting with eyes open for ninety seconds. Record changes without claiming that the exercise must work for everyone.

### Activity 43.3 — School consultation

Draft guidance for teachers on responding to a student who becomes blank and silent during confrontation.

## 16. Annotated Reading Pathways

**For foundations.** Begin with Ogden's sensorimotor work and Dana's clinically accessible polyvagal translation.

**For critique.** Read scientific critiques of polyvagal theory alongside Porges, so that clinical usefulness does not become uncritical certainty.

**For adolescents.** Combine somatic reading with developmental neuroscience, sleep medicine, pain assessment, and trauma-informed school practice.

**For ethics.** Study professional guidance on touch, consent, physical interventions, and work with minors before using body-based methods.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Somatic, Sensorimotor, and Polyvagal-Informed Practice** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is body-based approaches, autonomic regulation, and the neuroception of safety. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is safety, regulation, emotion, memory, relationships, and paced therapeutic work. The mechanism to keep in view is how threat, arousal, avoidance, meaning, and relational safety shape what a young person can feel, remember, and learn. The characteristic professional risk is moving into traumatic material before stabilisation, confusing calm with shutdown, or making disclosure the price of care. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 7. Apply the method | Use the chapter's core procedures in stabilisation sessions, trauma-informed schools and services, caregiver work, and specialist treatment pathways, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Nia is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Nia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Nia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on safety, regulation, emotion, memory, relationships, and paced therapeutic work. The first intervention is deliberately modest. Nia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Nia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Nia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be

sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Somatic, Sensorimotor, and Polyvagal-Informed Practice

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Somatic, Sensorimotor, and Polyvagal-Informed Practice** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Somatic, Sensorimotor, and Polyvagal-Informed Practice** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Somatic, Sensorimotor, and Polyvagal-Informed Practice**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a

discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Somatic, Sensorimotor, and Polyvagal-Informed Practice to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART VII

# Motivational, Social, Developmental, and Cultural Theories

Theories that attend to motivation, to social learning, to development, and to the cultural and gendered location of the person in the world.

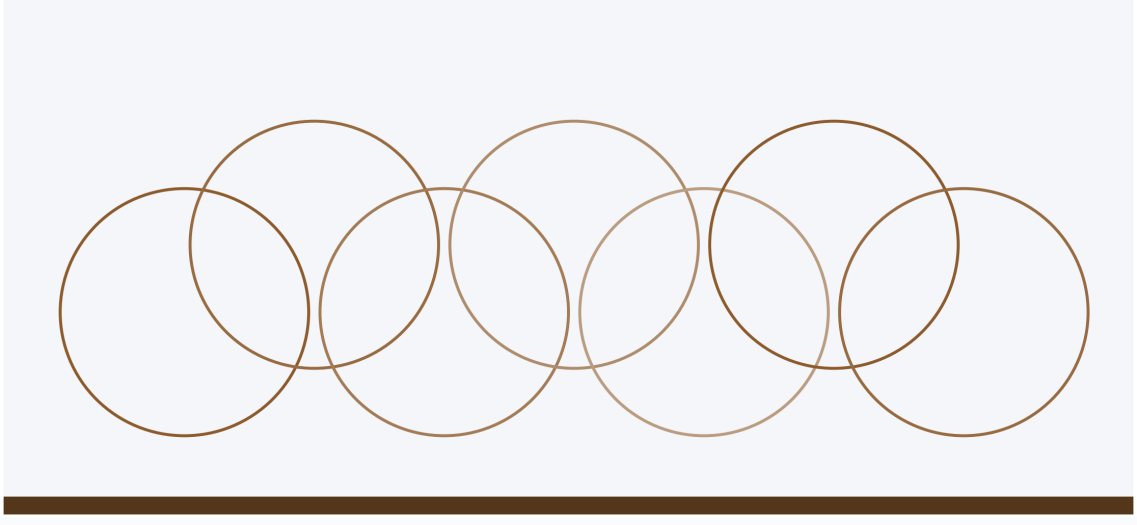
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CHAPTER 44

# Motivational Interviewing

Miller and Rollnick's four processes, change talk, and the adolescent adaptation of a conversational method.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain ambivalence and why direct persuasion often strengthens resistance.
- Describe the spirit of motivational interviewing: partnership, acceptance, compassion, and empowerment.
- Use open questions, affirmations, reflective listening, and summaries with adolescents.
- Elicit and strengthen change talk while softening sustain talk without arguing.
- Describe engaging, focusing, evoking, and planning processes.
- Apply motivational interviewing to school attendance, substance use, health behaviour, family conflict, and treatment engagement.
- Recognise ethical limits, especially with mandated and minor clients.

## Chapter Overview

Motivational interviewing (MI), developed by William R. Miller and Stephen Rollnick, is a collaborative conversation style for strengthening a person's own motivation and commitment to change. Its central insight is that ambivalence is normal. An adolescent may want to stop vaping because breathing and sport matter, and want to continue because vaping regulates stress and maintains belonging. A student may want qualifications and also want to avoid classrooms where failure feels public. A parent may want a calmer home and fear that reducing control means losing authority.

When practitioners argue for change, ambivalent people often argue back—not because they are irrational, but because conversation assigns positions. If the counsellor takes the “you must change” side, the young person may defend freedom, pleasure, identity, or scepticism. MI reverses the arrangement: the practitioner evokes the young person's own reasons, values, hopes, and confidence, then supports autonomy in deciding what to do.

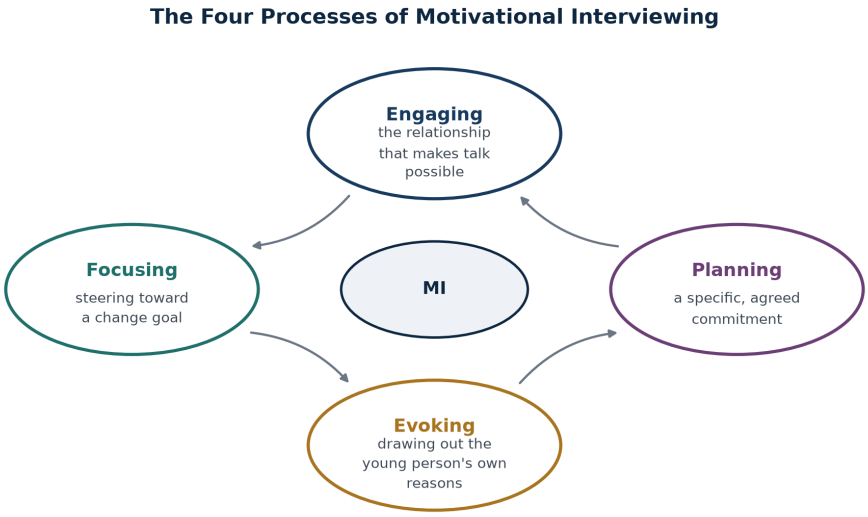



**Figure 44.32** The four processes of motivational interviewing: engaging, focusing, evoking, and planning, shown as a recursive cycle rather than a rigid staircase.

## 1. Development and Core Assumptions

MI emerged from addiction treatment in the 1980s, where Miller noticed that counsellor empathy predicted outcomes and that confrontational approaches often increased resistance. With Rollnick, the approach expanded into health care, corrections, education, parenting, and psychotherapy integration.

Four assumptions guide practice:

1. **Motivation is interpersonal.** It rises or falls within conversation.
2. **Ambivalence is not denial.** Mixed feelings contain useful information.
3. **People persuade themselves.** Statements spoken by the client—desire, ability, reasons, need, commitment—carry particular force.
4. **Autonomy is essential.** Change imposed without ownership is fragile and may be ethically unacceptable.

MI is not trickery to make a young person choose the adult's preferred outcome while feeling that it was their idea. That would violate its spirit. Transparency matters: the counsellor may state a concern, provide information with permission, and still respect the person's decision.

## 2. The Spirit of MI

Contemporary descriptions emphasise **partnership, acceptance, compassion, and empowerment**.

Partnership means working with, not on, the young person. Acceptance includes absolute worth, accurate empathy, autonomy support, and affirmation. Compassion places the young person's welfare above institutional convenience. Empowerment recognises existing strengths and resources rather than treating the client as empty until professionally filled.

The opposite spirit appears when the practitioner interrogates, labels, threatens, shames, or rushes to fix. Miller and Rollnick call the premature urge to solve the **fixing reflex**. With adolescents, the fixing reflex is amplified by adult responsibility: safeguarding, school targets, parental anxiety, and court or service mandates. MI does not remove those responsibilities; it changes how they are carried.

## 3. Change Talk and Sustain Talk

**Change talk** expresses movement toward change. Preparatory forms include desire (“I want”), ability (“I could”), reasons (“It would help”), and need (“I have to”). Mobilising forms include commitment (“I will”), activation (“I'm ready”), and taking steps (“I already tried”).

**Sustain talk** expresses movement away from change: pleasure, identity, fear, mistrust, practical barriers, loyalty to peers, or doubt. Sustain talk should not be crushed. It should be understood, reflected, and sometimes strategically explored so the young person can hear their own fuller position.

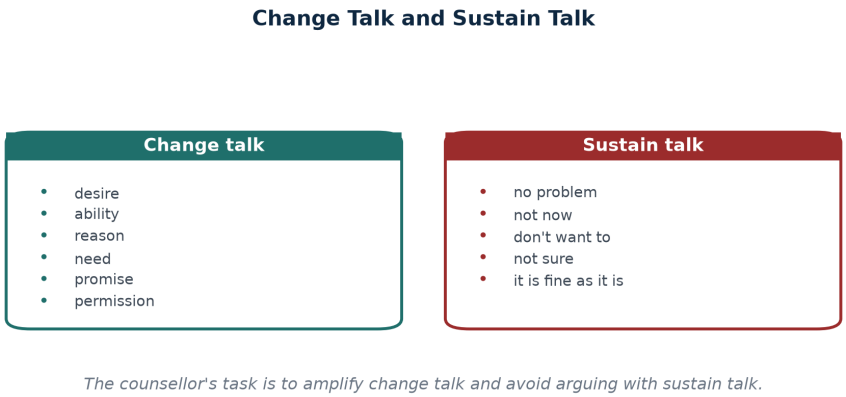



**Figure 44.33** Change talk and sustain talk as two sides of ambivalence, with counsellor reflections influencing which side becomes more elaborated.

A useful rule is that the more a young person argues for the status quo in session, the less likely change becomes. The counsellor therefore avoids supplying arguments that force the client into defence. Reflections often do more than questions because they allow the client to hear and revise their own meaning.

## 4. Core Skills: OARS

**Open questions** invite elaboration: “What concerns you most about how things are going?”

**Affirmations** recognise strengths and effort specifically: “You kept coming to appointments even though you expected to be lectured.”

**Reflective listening** forms the engine. Simple reflections repeat or slightly rephrase. Complex reflections add feeling, meaning, values, or the unstated side of ambivalence. A parent says, “He just doesn't care.” A reflection might be: “It's frightening to imagine he doesn't care, because then you feel powerless to reach him.” The aim is hypothesis, offered tentatively, not mind reading.

**Summaries** gather bouquets of change talk and offer them back: “You want your lungs back for football, you're tired of hiding devices from your mum, and you think weekdays might be easier than weekends. Did I miss anything?”

Advice is not forbidden. It is offered using elicit–provide–elicit: ask what the person knows and wants to know; offer information neutrally and with permission where required; elicit their interpretation and next step.

## 5. The Four Processes

**Engaging** establishes a working relationship. With adolescents, engagement includes explaining confidentiality, asking what would make sessions useful or unbearable, and noticing power differences.

**Focusing** identifies a direction. Several concerns may compete: attendance, cannabis, sleep, conflict, self-harm. Agenda mapping allows choice without pretending all can be solved at once.

**Evoking** draws out the young person's motivations, values, discrepancy, hopes, and confidence. This is the distinctive heart of MI.

**Planning** begins when motivation and confidence are sufficient. The practitioner supports a specific, chosen, realistic plan, anticipates barriers, and strengthens commitment without stealing ownership.

The processes are recursive. If planning produces resistance, return to evoking. If focus is unclear, return to engaging. Movement is not failure; it is information.

## 6. Further Reading for This Chapter


- Miller, W. R., & Rollnick, S. (2013). *Motivational Interviewing: Helping People Change* (3rd ed.).
- Miller, W. R., & Rollnick, S. (2023). *Motivational Interviewing: Helping People Change and Grow* (4th ed.).
- Rollnick, S., Kaplan, S. G., & Rutschman, R. (2016). *Motivational Interviewing in Schools*.
- Naar, S., & Suarez, M. (2021). *Motivational Interviewing with Adolescents and Young Adults* (2nd ed.).
- Dishion, T. J., & Kavanagh, K. (2003). *Intervening in Adolescent Problem Behavior: A Family-Centered Approach*.
- Arkowitz, H., Miller, W. R., & Rollnick, S. (Eds.). (2015). *Motivational Interviewing in the Treatment of Psychological Problems*.


## 7. Working with Adolescent Ambivalence

Adolescents are developing autonomy under conditions of continued dependence. Adults control money, transport, school access, health care, privacy, and legal status. An invitation to “choose” may therefore be experienced sceptically. The counsellor should be explicit about what is negotiable and what is not.

Safety requirements are not preferences to be evoked away. If a young person is at serious risk, the practitioner acts protectively, explains why, and preserves as much choice as possible inside the safety frame: who is told first, where a conversation happens, which trusted adult is present, and what the young person wants recorded.

Mandated clients require honesty. A school may require attendance at a programme; a court may require treatment; a parent may require counselling. MI distinguishes the mandated frame from the young person's goals within it. “You didn't choose to be referred. Given that you're here, what—if anything—would make this time useful to you?” This respects reality without pretending coercion is consent.

# 8. Key Techniques

## 8.1 Importance and confidence rulers

“On a scale from 0 to 10, how important is improving attendance?” Then ask why the number is not lower, which evokes strengths and reasons, and what would move it one point higher. Confidence rulers follow the same pattern. Avoid asking first why the number is not higher; that invites barriers and defence.

## 8.2 Looking back and looking forward

Looking back recalls life before the problem or previous successes. Looking forward imagines a preferred future and its practical details. These are not fantasies; they generate discrepancy between values and current trajectory.

## 8.3 Exploring values

Sport, loyalty, freedom, competence, family respect, creativity, faith-neutral personal commitments, independence, and being trusted often matter more than abstract health warnings. The counsellor listens for values without exploiting them manipulatively.

## 8.4 Double-sided reflections

“You want your mum off your back, and you also hate lying because it makes you feel trapped.” A double-sided reflection holds both sides and often ends with change talk so the young person can respond to it.

## 8.5 Emphasising autonomy

“It really is your decision” is powerful only when true within the stated frame. Used falsely, it becomes coercion with softer language.

# 9. Case Illustration: “I'm Not Bothered About School”

**Presentation.** Malik, fifteen, attends school two days a week. Staff describe him as capable but unmotivated. His mother wants the counsellor to “make him understand” that examinations determine his future.

**Engaging.** The counsellor begins: “You didn't choose this referral. What would make these meetings not a waste of your time?” Malik says, “If you don't lecture me.” The counsellor reflects, “You've had enough adults explaining your future at you.” He nods.

**Focusing.** Options are mapped: sleep, lessons, teachers, friends, coursework, home conflict. Malik chooses “mornings” as the least annoying focus.

**Evoking.** Importance of attending more is rated 4. Asked why not 1, Malik says he wants an apprenticeship and knows attendance is checked. Confidence is 3 because waking feels “impossible” and first lesson is with a teacher who publicly returns his work. Change talk emerges around competence and freedom: a reference from school would help him earn money and leave a neighbourhood conflict behind.

Sustain talk is explored respectfully. Staying home protects him from embarrassment and lets him help his grandmother. The counsellor reflects both sides rather than characterising home as avoidance.

**Information exchange.** With permission, the counsellor explains that the apprenticeship provider asks for attendance patterns but that late improvement still matters. Malik asks whether arriving for second lesson “counts for anything”; together they check school policy rather than speculate.

**Planning.** Malik chooses a two-week experiment: attend Tuesday to Friday from second lesson, sleep routine on school nights, phone charging downstairs on two nights he selects, and a private arrangement with the teacher to receive written feedback before any public comment. His mother agrees not to deliver morning lectures, replacing them with one agreed prompt and transport money placed by the door.

**Review.** Attendance rises to 70 per cent. More importantly, Malik describes himself as “testing whether I can do it”, not obeying the counsellor. When a setback follows a classroom humiliation, the process returns to evoking rather than blame.

## 10. Evidence and Research Support

Meta-analyses support MI for substance use and several health behaviours, with generally small to moderate effects that vary by outcome, fidelity, population, and comparison condition. Evidence with adolescents is promising for alcohol and substance use, treatment engagement, health behaviour, and some school applications, but heterogeneous.

MI is often brief and can enhance other treatments. It is not, by itself, sufficient treatment for severe depression, trauma, psychosis, eating disorders, or entrenched conduct problems. Effects decay when practitioners drift into persuasion, when fidelity is low, or when the environment makes chosen change practically impossible.

Training requires practice and feedback. Reading about empathy does not produce reflective listening. Audio review, coding, supervision, and coaching are associated with better skill retention than one-off workshops.

## 11. Strengths, Limitations, and Ethics

Strengths include respect for autonomy, usefulness at treatment entry, compatibility with other models, and attention to language as a mechanism of change. MI helps practitioners notice their own contribution to resistance.

Limitations include potential overfocus on individual choice where structural barriers dominate. A young person cannot choose safe housing into existence, make an inaccessible curriculum accessible, or remove discriminatory discipline by motivation alone. MI must therefore be paired with advocacy and practical support.

Ethically, practitioners must not use MI techniques to manufacture consent, conceal institutional goals, or evade safeguarding. Affirmations must be genuine. Reflections must not twist meaning. Summaries must

not erase sustain talk or dissent. The young person should know the counsellor's role, reporting obligations, and any non-negotiable requirements.

## 12. Key Terms

**Key Terms at a Glance**

- **Ambivalence** — Simultaneous reasons for and against change.
- **Change talk** — Client language favouring change.
- **Sustain talk** — Client language favouring the status quo or opposing change.
- **Fixing reflex** — The practitioner's premature urge to solve and persuade.
- **Elicit–provide–elicit** — A respectful sequence for exchanging information.
- **Agenda mapping** — Collaborative identification and prioritisation of possible change topics.
- **Epistemic humility in MI** — Recognition that the client is the primary expert on their values and life.

## 13. Chapter Summary

Motivational interviewing treats resistance less as a client trait than as a signal about the conversation. By partnering with adolescents, reflecting ambivalence, evoking their own reasons and values, and supporting feasible plans, the counsellor makes change speakable without making it compulsory. Its discipline is ethical as well as technical: influence must remain transparent, autonomy real, and safeguarding honest.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Why does arguing for change often strengthen sustain talk?
2. Distinguish preparatory from mobilising change talk.
3. Write a complex reflection for: “My mum goes on about vaping because she wants to control everything.”
4. Why ask why an importance rating is not lower before asking what would raise it?
5. How should MI be adapted for a mandated adolescent client?
6. When is advice-giving compatible with MI?
7. Identify one structural barrier that MI alone cannot solve.

## 15. Practice Activities

**Practice and Application**

### Activity 44.1 — Reflection practice

Convert five closed questions into open questions and complex reflections.

### Activity 44.2 — Ruler interview

In pairs, use importance and confidence rulers about a low-stakes habit. Practise asking why the number is not lower.

### Activity 44.3 — Spirit audit

Review a fictional transcript and mark partnership, acceptance, compassion, empowerment, and the fixing reflex.

## 16. Annotated Reading Pathways

**For foundations.** Miller and Rollnick's fourth edition is the most current comprehensive statement.

**For adolescents.** Naar and Suarez provide developmental and family adaptations, including work with caregivers.

**For schools.** Rollnick and colleagues show how brief MI conversations can be used without turning teachers into therapists.

**For evidence.** Read meta-analyses critically, noting fidelity measurement and whether MI was stand-alone or an engagement prelude.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Motivational Interviewing** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is miller and Rollnick's four processes, change talk, and the adolescent adaptation of a conversational method. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture, relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Marcus is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Marcus's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The

practitioner first establishes safety, consent, and context. Marcus explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Marcus chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Marcus has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Marcus can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.

- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Motivational Interviewing

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Motivational Interviewing** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a

recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Motivational Interviewing** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

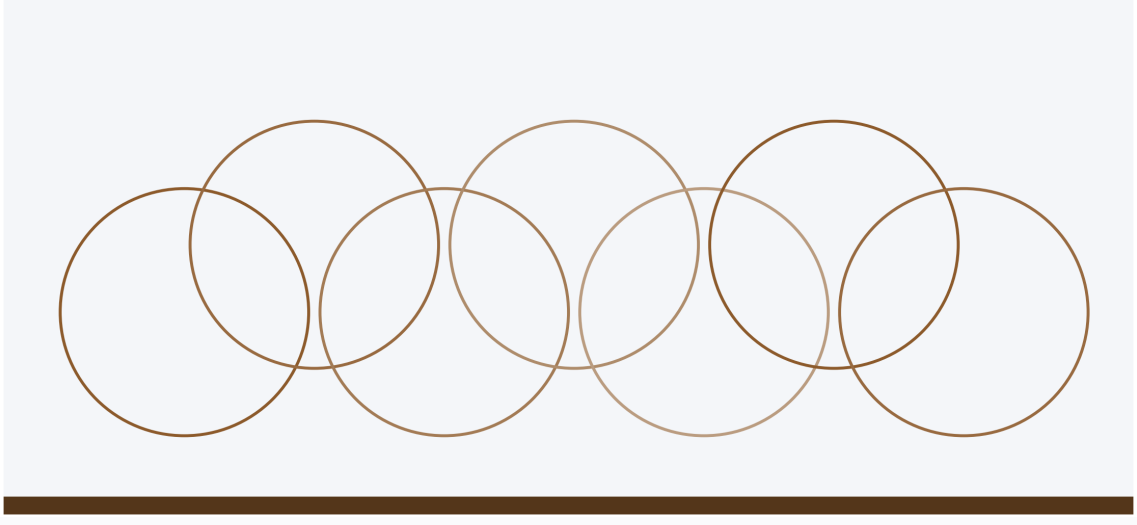
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Motivational Interviewing**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Motivational Interviewing to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 45

# Social Cognitive Learning Theory

Bandura's reciprocal determinism, self-efficacy, and the modelling of behaviour in social contexts.



## Learning Objectives

By the end of this chapter, the student should be able to:

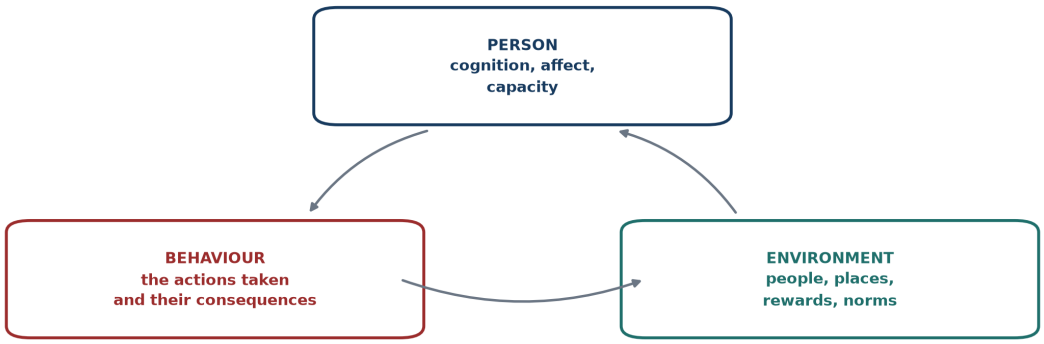
- Explain Bandura's move from social learning theory to social cognitive theory.
- Describe reciprocal determinism among person, behaviour, and environment.
- Define self-efficacy and identify its four sources.
- Distinguish outcome expectations, goals, self-regulation, and observational learning.
- Apply social cognitive theory to academic behaviour, peer influence, aggression, health, digital life, and therapeutic change.
- Design interventions that build mastery rather than merely encourage confidence.
- Evaluate limitations, cultural considerations, and ethical concerns.

## Chapter Overview

Albert Bandura transformed learning theory by insisting that people are not merely shaped by rewards and punishments. They observe, anticipate, interpret, set goals, judge their capabilities, regulate themselves, and alter the environments that alter them. Social cognitive theory therefore bridges behavioural and cognitive traditions while retaining a strong interest in social context.

For adolescent counselling, the theory is exceptionally practical. Young people learn from peers, siblings, influencers, teachers, fictional characters, and adults who may not know they are modelling. They avoid tasks not only because they lack skill but because they expect humiliation. They repeat behaviour because it produces status. They may know what to do yet lack confidence, opportunity, feedback, or a setting in which the behaviour is safe.

**Reciprocal Determinism: Person, Behaviour, and Environment**




*The three influences act on one another continuously; none is sufficient alone.*

**Figure 45.34** Reciprocal determinism: personal factors, behaviour, and environment continually influence one another.

## 1. From Modelling to Agency

Bandura's early experiments demonstrated observational learning: children who saw a model behave aggressively could reproduce novel aggressive acts, especially when the model was rewarded or held status. Later theory expanded beyond imitation. Human agency involves intentionality, forethought, self-reactiveness, and self-reflectiveness.

People act as contributors to their own development. But agency is not unlimited free will. A young person in poverty, unsafe housing, discriminatory schooling, or coercive relationships has real constraints. Social cognitive theory is strongest when the reciprocal model includes those constraints rather than converting every outcome into a test of personal belief.

## 2. Reciprocal Determinism

Personal factors (beliefs, affect, biology, knowledge), behaviour, and environment influence one another bidirectionally.

A student believes she is “bad at maths” (person). She avoids practice and hides confusion (behaviour). The teacher gives less feedback and peers become the audience for jokes about failure (environment). The belief strengthens. Intervention can enter at any point: mastery practice, teacher feedback routines, peer grouping, success experiences, or reinterpretation of difficulty.

This model prevents simplistic counselling. Telling the student to “believe in yourself” addresses one corner while leaving task design and classroom humiliation intact.

## 3. Observational Learning

Observational learning requires attention, retention, reproduction, and motivation.

**Attention** depends on salience, status, similarity, emotional arousal, and complexity. Adolescents often attend more to peers and online creators than to adults.

**Retention** involves symbolic coding and rehearsal. A young person may remember a humiliating comeback word for word and forget safety instructions.

**Reproduction** requires capability and opportunity. Watching assertiveness is not the same as having language, timing, and social permission to use it.

**Motivation** depends on observed and expected consequences. If aggression produces laughter and status, moral instruction alone competes poorly.

Counselling can therefore select models carefully: credible peers, coping models who struggle and improve, video self-modelling, teacher modelling of repair, and parent modelling of calm disagreement.

## 4. Self-Efficacy

Self-efficacy is belief in one's capability to organise and execute actions required for a specific task in a specific context. It is not general self-esteem. A student may have high efficacy for football and low efficacy for speaking in class.

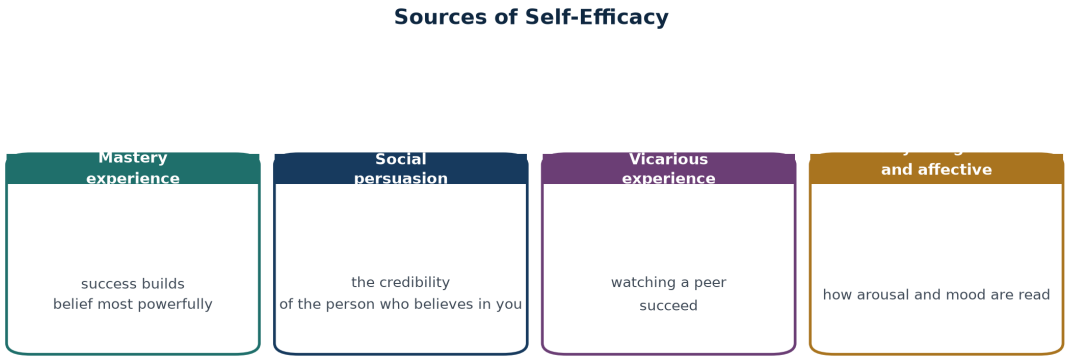



**Figure 45.35** Sources of self-efficacy: mastery experiences, vicarious experience, verbal persuasion, and physiological or affective states.

### 4.1 Mastery experiences

Successful performance is the strongest source, especially when success follows effort on a meaningful challenge and is attributed to strategy and capability. Tasks must be graded. Artificially easy praise produces fragile confidence.

### 4.2 Vicarious experience

Seeing someone similar succeed raises efficacy; seeing similar peers fail can lower it. Similarity matters: age, background, disability, gender, prior attainment, and social identity may affect identification.

### 4.3 Verbal persuasion

Encouragement helps when credible, specific, and paired with attainable steps. Empty reassurance can backfire after failure.

### 4.4 Physiological and affective states

Anxiety, fatigue, pain, excitement, and calm are interpreted as information about capability. Young people can learn to distinguish arousal from inability: a racing heart before speaking may signal importance, not inevitable failure.

## 5. Outcome Expectations, Goals, and Self-Regulation

People ask not only “Can I do it?” but “What will happen if I do?” Positive outcome expectations may involve grades, status, safety, belonging, money, freedom, or adult trust. Negative expectations may involve ridicule, punishment, exclusion, or loss of identity.

Goals organise behaviour when they are specific, challenging but attainable, proximal, and connected to feedback. Distal goals such as “get into college” rarely regulate a tired Tuesday evening. Proximal

goals—complete two problems, attend first lesson, revise for fifteen minutes—create mastery information.

Self-regulation includes self-observation, judgement against standards, and self-reaction. Diaries, apps, checklists, teacher feedback, and self-graphing make progress visible. Standards should be developmental and humane; perfectionistic self-monitoring can become another source of shame.

# 6. Further Reading for This Chapter


- Bandura, A. (1977). *Social Learning Theory*.
- Bandura, A. (1986). *Social Foundations of Thought and Action*.
- Bandura, A. (1997). *Self-Efficacy: The Exercise of Control*.
- Schunk, D. H., & DiBenedetto, M. K. (2020). Motivation and social cognitive theory. *Contemporary Educational Psychology, 60*, 101832.
- Pajares, F. (1996). Self-efficacy beliefs in academic settings. *Review of Educational Research, 66*(4), 543–578.


# 7. Clinical and Educational Applications

## 7.1 Academic avoidance

A social cognitive formulation asks:

- What specific task efficacy is low?
- What outcomes are expected from trying and failing?
- Which models dominate?
- What mastery opportunities exist?
- How do teachers and peers respond to error?
- Which self-regulation tools are available?
- What environmental barriers can be changed?

Intervention might include graded tasks, worked examples, peer coping models, private feedback before public performance, strategy attribution (“the plan worked”), and visible progress records.

## 7.2 Aggression and peer status

Aggression may be maintained by efficacy beliefs (“I can make them back down”), outcome expectations (“respect requires retaliation”), modelling, moral disengagement, and environments that reward dominance. Intervention must change status economies, not only teach anger facts. Schools can create legitimate routes to competence and recognition, use restorative processes where safe, involve credible peer models, and ensure adults do not model humiliation.

## 7.3 Health and digital behaviour

Young people observe sleep, eating, exercise, vaping, sexual behaviour, and risk through families, peers, and algorithmic media. Social cognitive work identifies models, outcome expectations, and environmental

cues. A plan to reduce late-night phone use is stronger when it alters charging location, peer response norms, replacement activities, and morning goals than when it relies on warnings.

### 7.4 Therapeutic change

Many therapies work partly by increasing efficacy. Exposure creates mastery. Skills training provides models and rehearsal. Behavioural activation generates success experiences. Parent training increases parental efficacy. MBT improves confidence that mental states can be understood. Social cognitive theory therefore integrates mechanisms across this book.

## 8. Case Illustration: The Student Who Would Not Present

**Presentation.** Priya, thirteen, refuses oral presentations. She is articulate with friends and writes well. She says, “I'm just not a confident person.”

**Formulation.** Her presentation-specific efficacy is low after freezing during a primary-school assembly. She expects peers to laugh and teachers to think she is lazy. She watches confident classmates and concludes they are naturally different. Physiological arousal is interpreted as proof she will forget everything. Avoidance prevents corrective mastery.

**Intervention.** The counsellor and teacher design a mastery ladder: record a thirty-second voice note; present to the counsellor; present to one trusted friend; read one paragraph to a small group; answer one prepared question; present two slides. A coping model video shows a similar student pausing, breathing, and recovering after losing place. Feedback identifies strategies used, not personality traits. Arousal is reframed as activation that can coexist with performance.

Environmental changes matter. The teacher stops surprise questioning during the ladder, permits cue cards, and arranges the first small-group audience. Peers are given roles as summariser and questioner rather than evaluators.

**Outcome.** Priya completes a three-minute group presentation. She remains nervous and does not become “a confident person” in general. Her belief becomes precise: “I can present if I prepare, start small, and know how to recover.” That precision is the therapeutic achievement.

## 9. Evidence and Limitations

Self-efficacy predicts academic performance, persistence, health behaviour, career exploration, and treatment outcomes across many studies. Interventions that create mastery and credible modelling can change efficacy and behaviour, though effects vary and reciprocal causality makes simple interpretation difficult: success raises efficacy, and efficacy supports success.

Observational learning is robustly established, but popular claims about media effects require nuance. Exposure to aggressive or risky models interacts with prior disposition, family context, peer norms, identification, reward, and opportunity. Social cognitive theory does not justify moral panic or censorship by itself.

Limitations include measurement by self-report, cultural variation in agency and selfhood, and risk of blaming individuals for structural constraints. In collectivist contexts, efficacy may be experienced as

collective efficacy—"we can do this"—rather than solitary confidence. Interventions should therefore assess family, peer, school, and community efficacy.

## 10. Cultural and Ethical Considerations

Models are not neutral. Representation matters: who is shown succeeding, whose behaviour is labelled leadership or defiance, and which futures appear imaginable. Counsellors should use diverse, credible models and avoid tokenism.

Verbal persuasion must not become pressure. Encouraging a young person to attempt an unsafe disclosure, confront an abuser, or perform beyond disability-related capacity in the name of confidence is unethical. Mastery must be designed with accessibility, safeguarding, and realistic resources.

Digital modelling raises consent issues. Video self-modelling and peer examples require permission, secure storage, and attention to embarrassment. Public progress charts can motivate some students and humiliate others; private feedback should be default where stigma is likely.

## 11. Key Terms

**Key Terms at a Glance**

- **Reciprocal determinism** — Bidirectional influence among person, behaviour, and environment.
- **Observational learning** — Learning through attention, retention, reproduction, and motivation.
- **Self-efficacy** — Task- and context-specific belief in one's capability to execute required actions.
- **Mastery experience** — Successful performance that provides evidence of capability.
- **Collective efficacy** — A group's shared belief in its capacity to act effectively.
- **Outcome expectation** — Belief about likely consequences of a behaviour.
- **Coping model** — A model who visibly struggles, corrects errors, and improves.

## 12. Chapter Summary

Social cognitive theory explains why young people do not simply do what they know. They watch, infer, anticipate, judge capability, pursue status and safety, and act within environments that answer back. Effective counselling changes more than belief: it engineers mastery, selects models, alters consequences, teaches self-regulation, and removes barriers.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Apply reciprocal determinism to school refusal.
2. Why is self-efficacy not the same as self-esteem?
3. Rank the four sources of efficacy for a student returning after long absence, and justify the ranking.
4. What makes a coping model more useful than a flawless model?
5. Identify one intervention each at the person, behaviour, and environment points of a case.
6. How can collective efficacy change an intervention plan?
7. State one ethical risk of video modelling with adolescents.

## 14. Practice Activities

**Practice and Application**

### Activity 45.1 — Efficacy ladder

Choose a specific avoided task and design six mastery steps with feedback and attribution at each step.

### Activity 45.2 — Model audit

List the five most influential models in a fictional adolescent's digital and offline life. Identify what each model teaches and what consequences follow.

### Activity 45.3 — Reciprocal map

Draw person–behaviour–environment influences for aggression maintained by peer status.

## 15. Annotated Reading Pathways

**For theory.** Bandura (1986) remains the comprehensive source; Bandura (1997) is essential for self-efficacy.

**For education.** Pajares and Schunk connect efficacy to classroom design, feedback, and achievement.

**For practice.** Translate every encouragement into a mastery opportunity, credible model, or environmental change. If none exists, the intervention is probably only exhortation.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Social Cognitive Learning Theory** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is bandura's reciprocal determinism, self-efficacy, and the modelling of behaviour in social contexts. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture,

relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Leah is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Leah's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Leah explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Leah chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Leah has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Leah can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Social Cognitive Learning Theory

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Social Cognitive Learning Theory** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Social Cognitive Learning Theory** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

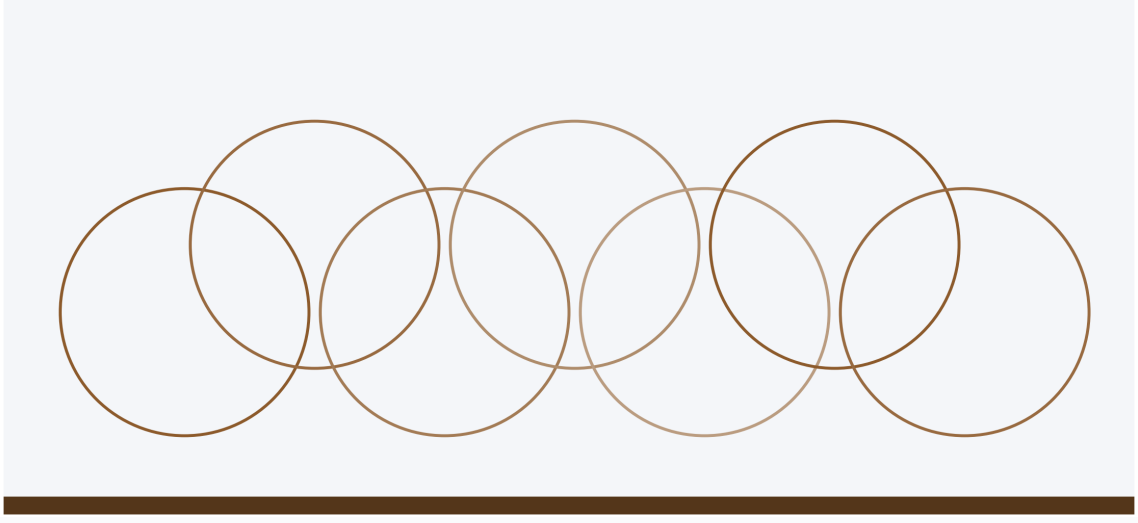
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Social Cognitive Learning Theory**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Social Cognitive Learning Theory to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 46

# Adolescent Development: Theory and Counseling Implications

Identity, autonomy, risk, and peer relations as a developmental framework for intervention.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe adolescent development across biological, cognitive, emotional, social, moral, and identity domains.
- Explain puberty, brain development, sleep changes, and heightened reward sensitivity without reducing adolescence to hormones or defective decision-making.
- Compare major developmental theories and identify their counselling implications.
- Explain identity formation, autonomy, peer belonging, family renegotiation, sexuality, and digital development.
- Recognise diversity in developmental pathways, including disability, chronic illness, migration, poverty, care experience, and cultural variation.
- Formulate presenting problems developmentally rather than treating adolescence itself as pathology.
- Apply developmental knowledge to assessment, timing, confidentiality, family work, and school consultation.

## Chapter Overview

Adolescence is not a waiting room between childhood and adulthood. It is a developmental period with its own tasks, sensitivities, opportunities, and risks. Young people are biologically maturing, thinking more abstractly, forming identity, renegotiating family dependence, seeking peer belonging, exploring sexuality and values, and preparing for adult roles—all while adults still control many conditions of their lives.

Counsellors who misunderstand development make predictable errors. They treat normal autonomy seeking as oppositional pathology; interpret concrete thinking as resistance; expect adult-like emotional regulation from a sleep-deprived fourteen-year-old; disclose information to parents as if the client were eight; or impose independence as if the client were twenty-five. Developmental formulation protects against both infantilisation and premature adultification.

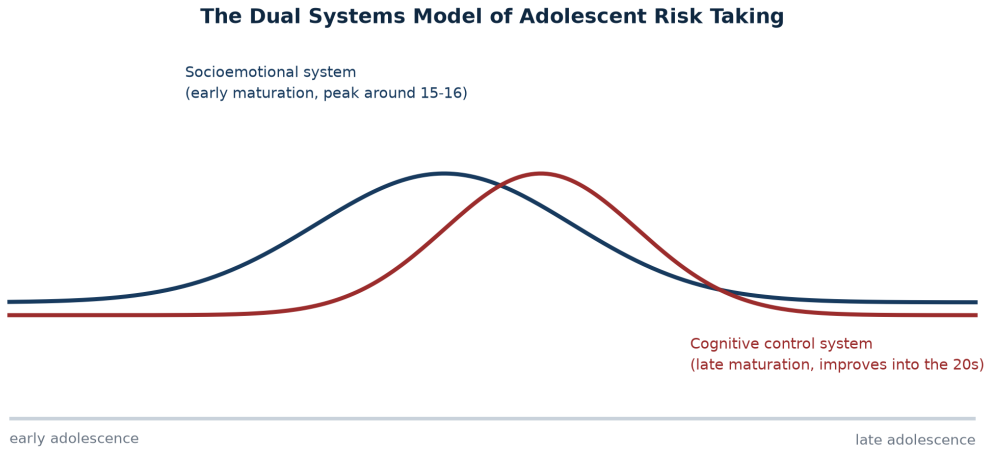



**Figure 46.36** Adolescent dual-systems development: heightened reward and peer sensitivity alongside gradually maturing cognitive control, with context determining balance.

# 1. What Is Adolescence?

Chronological boundaries vary. Many contemporary accounts describe adolescence as approximately ten to twenty-four, recognising extended education, later financial independence, and continued brain development. Legal, cultural, educational, and family transitions do not align neatly. A married seventeen-year-old worker, a thirteen-year-old in secondary school, and a twenty-two-year-old university student may all be called adolescents or young people while facing very different tasks.

Development is also asynchronous. Puberty may be early while emotional regulation is young; abstract reasoning may be strong while social confidence is fragile; sexual maturity may precede access to accurate information or safe relationships. Assessment should ask, domain by domain, “Where is this young person developmentally?” rather than assume age answers the question.

# 2. Biological Development

## 2.1 Puberty

Puberty involves hormonal changes, growth, sexual maturation, body hair, skin changes, menstruation, voice change, and altered body composition. Timing matters. Early or late maturation can affect peer status, adult expectations, body image, sexualisation, and risk. Adults may treat a physically mature child as emotionally older, a pattern sometimes called adultification, with serious safeguarding implications.

Counsellors should ask sensitively about pubertal concerns, menstrual health, pain, acne, height, weight change, breast or genital development, and embarrassment in changing rooms or sport. Medical referral is appropriate where pain, delayed or precocious development, eating concerns, or distress require assessment.

## 2.2 Brain development

Adolescent brain development includes synaptic pruning, myelination, changing reward sensitivity, and gradual strengthening of prefrontal systems involved in planning, inhibition, and long-term consequence evaluation. The popular “dual systems” account contrasts heightened socioemotional and reward sensitivity with still-maturing cognitive control.

This model is useful but incomplete. Adolescents can reason well in calm, supported conditions and make poorer decisions when peers are present, emotions are high, time is short, or rewards are immediate. Adults do the same, albeit often less intensely. The implication is not that adolescents are incapable; it is that environments should reduce unnecessary heat: sleep, time to decide, private rather than public correction, limited audience, and adults who do not force instant answers during conflict.

## 2.3 Sleep

Puberty shifts circadian preference later while school often starts early. Sleep deprivation impairs mood, learning, impulse control, pain tolerance, and family patience. Assessment that ignores sleep is incomplete. Counsellors should ask about bedtime, waking, naps, screens, caffeine and energy drinks, shift-working parents, shared rooms, nightmares, snoring, and weekend catch-up patterns.

Sleep intervention is developmental work: regular wake time, morning light, reduced late caffeine, planned screen transitions, treatment of anxiety or medical problems, and school advocacy where start times or travel make adequate sleep impossible.

## 3. Cognitive Development

Adolescents increasingly use abstract, hypothetical, and metacognitive thought. They can imagine alternative selves, question rules, detect hypocrisy, and reason about justice. This is not merely a risk; it is an intellectual achievement.

At the same time, abstract capacity is uneven. A young person may debate politics brilliantly and think concretely about contraception, debt, or online permanence. Stress narrows cognition. Neurodevelopmental conditions, trauma, language development, and educational opportunity shape expression.

Counselling should therefore check understanding rather than infer it from vocabulary. Ask the young person to explain a plan in their own words. Use examples, drawings, scaling, timelines, and written summaries. Do not mistake fluent therapy language for integrated understanding.

## 4. Emotional Development

Emotional intensity, self-consciousness, and sensitivity to evaluation increase. Shame becomes a powerful organising emotion because identity and peer status are at stake. Emotional regulation develops through co-regulation, practice, sleep, language, and environments that do not continually overwhelm.

The adolescent may oscillate between wanting comfort and rejecting it. A parent offers a hug; the young person recoils, then later sits nearby while pretending to watch television. Counsellors and parents need to read proximity flexibly and not demand that care be accepted in adult-preferred forms.

Emotional development is also cultural. Families differ in expression, restraint, humour, privacy, and help seeking. Assessment should ask what feelings are acceptable at home, which are hidden, and what happens when someone cries, protests, or becomes quiet.

## 5. Identity Development

Erikson placed identity versus role confusion at the centre of adolescence. Contemporary accounts are less linear. Identity includes ethnicity, culture, language, gender, sexuality, disability, faith-neutral value commitments where personally chosen, class, nationality, family story, interests, body, and future occupation. Exploration and commitment interact over time, and identity may be revisited throughout life.

Marcia described statuses based on exploration and commitment: diffusion, foreclosure, moratorium, and achievement. These are heuristics, not diagnoses. A young person may be exploratory about career, foreclosed about family values, and uncertain about gender or sexuality. Safety affects exploration: it is not developmental failure when a young person conceals identity in an unsafe home or community.

Counselling supports identity by providing language, mirrors, time, and non-possessive curiosity. The counsellor does not assign an identity, rush disclosure, or treat questioning as a phase to be corrected.

## 6. Further Reading for This Chapter

- Steinberg, L. (2023). *Adolescence* (13th ed.).
- Arnett, J. J. (2024). *Adolescence and Emerging Adulthood: A Cultural Approach*.
- Lerner, R. M., & Steinberg, L. (Eds.). (2009). *Handbook of Adolescent Psychology*.

- Dahl, R. E., Allen, N. B., Wilbrecht, L., & Suleiman, A. B. (2018). Importance of investing in adolescence from a developmental science perspective. *Nature, 554*, 441–450.
- Blakemore, S.-J. (2018). *Inventing Ourselves: The Secret Life of the Teenage Brain*.
- Erikson, E. H. (1968). *Identity: Youth and Crisis*.


## 7. Family Autonomy and Relatedness

Healthy adolescent development is not separation from family but transformation of family relationships. Autonomy grows best with relatedness: the young person can disagree, make choices, and spend time elsewhere while retaining connection and support.

Parents often experience ordinary autonomy as loss or disrespect. The adolescent experiences parental protection as control or distrust. Both may be partly right. Counselling helps families distinguish safety rules from habits of childhood, negotiate graduated freedoms, and repair after conflict.

Key questions include:

- Which decisions does the young person already make well?
- Which freedoms are linked to demonstrated skills rather than age alone?
- What information do parents genuinely need for safety?
- What privacy is developmentally appropriate?
- How are mistakes handled—by teaching and proportionate consequence, or by total withdrawal of trust?

Families under stress may have fewer resources for gradual negotiation. Poverty, unsafe neighbourhoods, parental illness, migration, and caring responsibilities can require adolescents to mature early in some domains while remaining dependent in others.

## 8. Peers, Romance, and Digital Life

Peer belonging is a developmental need, not a distraction from real life. Friendship teaches reciprocity, loyalty, repair, status, intimacy, and boundary setting. Peer rejection and bullying can therefore have profound effects.

Romantic and sexual development involves curiosity, attraction, consent, communication, body image, and values. Counsellors should provide non-shaming, medically accurate, safeguarding-aware practice and know local law and referral pathways. The counsellor does not facilitate secrecy that places a young person at risk, nor disclose lawful private information merely to satisfy adult anxiety.

Digital environments are social environments. Group chats, gaming, social platforms, image sharing, pornography, influencers, and algorithmic recommendation shape identity and comparison. Assessment should ask what platforms do for the young person—belonging, creativity, learning, escape, status—as well as what they cost. A purely confiscatory response may remove support as well as risk.

## 9. School and Future Development

School is simultaneously an educational institution, peer society, evaluation system, and route to future opportunity. Academic identity forms through feedback, grouping, teacher relationships, curriculum

relevance, language, disability access, and experiences of success.

Developmental counselling asks whether demands match capacity and support. A student may fail because work is too difficult, too easy, inaccessible, publicly humiliating, poorly timed for sleep, or irrelevant to any imaginable future. Motivation cannot be separated from curriculum and relationship.

Career development begins long before formal choices. Young people need exposure to possibilities, work experience, adults who imagine futures for them, and freedom from premature narrowing based on class, gender, race, disability, or school predictions.

## 10. Developmental Formulation in Counselling

A developmental formulation includes:

1. pubertal and health context;
2. sleep, energy, pain, and medication;
3. cognitive and language capacities;
4. emotional regulation and co-regulation;
5. identity exploration and safety;
6. family autonomy–relatedness balance;
7. peer belonging and digital life;
8. school fit and future orientation;
9. culture, migration, faith-neutral values where relevant, and community resources;
10. strengths, interests, and developmental opportunities.

The formulation should lead to timing decisions. Some work is best done individually, some with parents, some through school change, and some deferred until sleep, safety, or stability improves. A developmentally appropriate plan does not demand that a young person solve in therapy what adults must solve in the environment.

## 11. Case Illustration: “Lazy, Rude, and Always on That Phone”

**Presentation.** Daniel, fourteen, is referred for oppositional behaviour. He sleeps until noon at weekends, argues when woken, ignores homework, and becomes angry when his phone is removed at midnight.

**Developmental assessment.** Daniel's circadian rhythm has shifted later; school requires waking at 6:15 a.m. He is chronically sleep deprived. Puberty has made him highly sensitive to public correction; two teachers use sarcasm. His phone is not merely entertainment but the location of his friendship group after his best friend moved school. At home he is given few choices but is expected to show adult self-control when tired.

**Formulation.** The presenting behaviour is interaction among sleep biology, peer belonging, autonomy needs, family control patterns, and school humiliation—not a unitary oppositional trait.

**Intervention.** The family negotiates a sleep plan: fixed wake time, phone charging outside the bedroom after an agreed time, but protected group-chat time earlier in the evening. His mother replaces repeated morning calls with one prompt, light, breakfast, and a predictable departure routine. School agrees to private correction and a seating change. Daniel chooses a football activity that restores peer contact offline.

Daniel remains argumentative at times, especially when tired. The difference is that adults stop building a character diagnosis from predictable developmental friction and start changing timing, audience, and choices.

## 12. Evidence and Critical Considerations

Developmental science supports the importance of sleep, puberty timing, peer context, family autonomy support, school belonging, and sensitive periods for learning and stress. Brain research informs practice but should not be oversimplified into claims that adolescents cannot reason or that a brain scan can determine individual maturity.

Stage theories can become rigid. Young people do not pass through identical sequences at identical ages, and cultural pathways to adulthood differ. Competence may be domain-specific. A young parent, carer, worker, or migrant adolescent may hold adult responsibilities while still needing adolescent protection and support.

Adultification and infantilisation are both harmful. Adultification denies protection to young people perceived as older, often along racialised and gendered lines. Infantilisation denies agency and privacy. Ethical assessment continually asks: What protection does this young person need? What agency can they exercise? What support makes both possible?

## 13. Key Terms

**Key Terms at a Glance**

- **Adultification** — Treating a child or adolescent as older and less in need of protection than they are.
- **Autonomy–relatedness** — The developmental balance between self-direction and continuing family connection.
- **Circadian shift** — Puberty-associated tendency toward later sleep and waking times.
- **Developmental formulation** — Explanation of difficulties in relation to developmental capacities, tasks, contexts, and timing.
- **Dual-systems model** — Account of adolescence emphasising reward sensitivity and maturing cognitive control.
- **Identity exploration** — Active consideration of values, roles, identities, and possible futures.
- **School belonging** — The experience of being accepted, respected, and able to participate in school life.

## 14. Chapter Summary

Adolescent counselling begins when adults stop asking only, “How do we stop this behaviour?” and ask also, “What developmental work is this young person attempting, under what biological, relational, cultural, and institutional conditions?” Development does not excuse harm, but it explains timing, sensitivity, and the kinds of support that make change possible.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain why adolescence should not be treated as either childhood or adulthood.
2. How can the dual-systems model be used without stereotyping adolescents?
3. Identify four ways sleep deprivation can masquerade as behavioural disorder.
4. Distinguish autonomy from separation.
5. Why is peer belonging a developmental need rather than a reward?
6. Write a developmental formulation for a fifteen-year-old school refuser.
7. Give one example each of adultification and infantilisation in services.

## 16. Practice Activities

**Practice and Application**

### Activity 46.1 — Developmental map

Create a domain-by-domain developmental map for a fictional fifteen-year-old: body, sleep, cognition, emotion, identity, family, peers, digital life, and school.

### Activity 46.2 — Environment redesign

Take a common conflict about homework and redesign timing, audience, choice, feedback, and sleep rather than focusing only on motivation.

### Activity 46.3 — Language audit

Rewrite five adult statements that label an adolescent as lazy, manipulative, dramatic, or attention-seeking into developmental observations.

## 17. Annotated Reading Pathways

**For comprehensive development.** Steinberg and Arnett provide complementary biological and cultural accounts.

**For brain development.** Blakemore is accessible and careful; use primary neuroscience reviews to avoid pop-neuroscience claims.

**For identity.** Erikson and Marcia are historically important but should be read alongside contemporary work on culture, gender, sexuality, disability, and migration.

**For practice.** Translate every developmental claim into an assessment question, an environmental adjustment, or a timing decision.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Adolescent Development: Theory and Counseling Implications** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is identity, autonomy, risk, and peer relations as a developmental framework for intervention. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture, relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the

result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tobias is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tobias's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tobias explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Tobias chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tobias has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tobias can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by

participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?

- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Adolescent Development: Theory and Counseling Implications

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Adolescent Development: Theory and Counseling Implications** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Adolescent Development: Theory and Counseling Implications** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Adolescent Development: Theory and Counseling Implications**, and the response of the

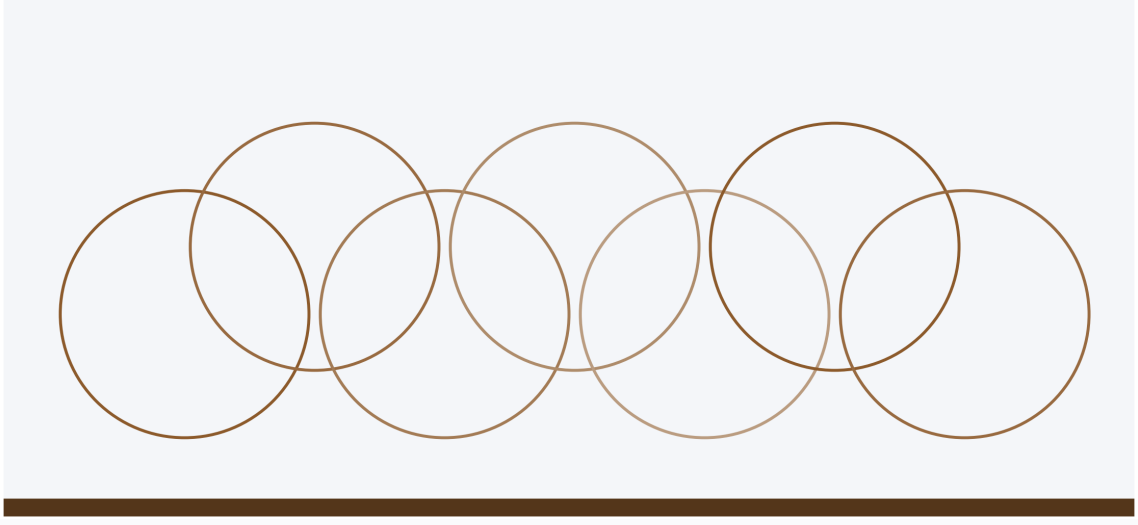
surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Adolescent Development: Theory and Counseling Implications to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 47

# Psychodynamic and Object Relations Approaches

The relational substrate of personality, transference, and developmental lines of interpretation.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the development of psychodynamic thinking from Freud through ego psychology, object relations, self psychology, and relational approaches.
- Explain unconscious processes, conflict, defence, transference, countertransference, and enactment in clinically usable language.
- Describe object relations concepts: internal objects, splitting, projection, projective identification, and containment.
- Apply psychodynamic formulation to adolescent identity, family relationships, school behaviour, and therapeutic ruptures.
- Distinguish evidence-informed psychodynamic practice from unfalsifiable interpretation.
- Recognise cultural, ethical, and safeguarding limits.

## Chapter Overview

Psychodynamic approaches ask how earlier relationships, conflicted wishes and fears, defences, and unconscious patterns continue to organise present experience. Their language can appear remote from school corridors and family kitchens, yet their central observations are familiar. A student experiences a neutral teacher as contemptuous. A parent reacts to a fifteen-year-old as if she were still seven—or as if she were the parent's own critical mother. A counsellor feels an inexplicable urge to rescue, punish, withdraw, or become unusually helpless. Psychodynamic practice slows these moments down and asks what relational pattern is being repeated, avoided, or communicated.

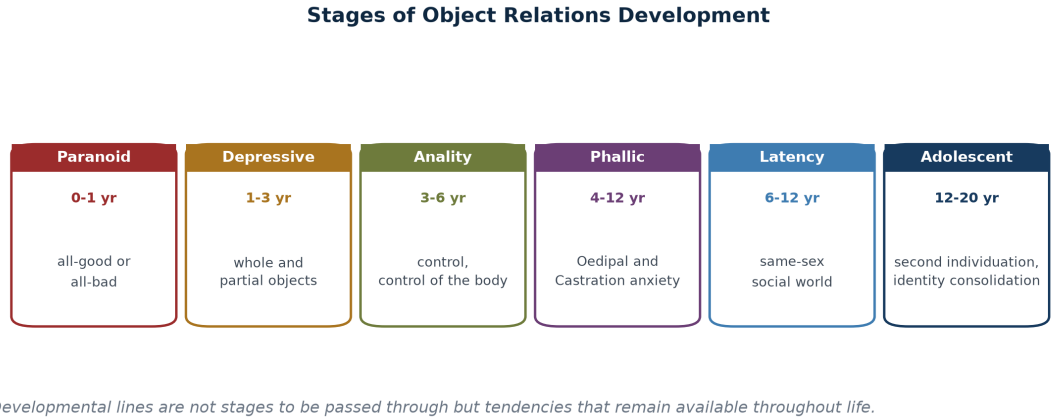



**Figure 47.37** Object relations patterns: early relational experiences become internal templates that shape expectations of self and other.

## 1. Historical Development

Freud's work introduced the ideas that mental life includes unconscious processes, that symptoms may express conflict, that childhood experience matters, and that the therapeutic relationship itself becomes a

field in which old patterns appear. Many specific Freudian claims have been revised or rejected, but the clinical attention to hidden meaning, ambivalence, defence, and repetition remains influential.

Ego psychology emphasised adaptation, defence analysis, and strengths of the ego. Object relations theorists—including Klein, Fairbairn, and Winnicott—shifted attention toward internalised relationships: the self in relation to “objects”, meaning significant others as represented in the mind. Self psychology, associated with Kohut, emphasised empathy, self-cohesion, and needs for mirroring, idealisation, and belonging. Relational and intersubjective approaches later stressed that therapy is co-created by two subjectivities rather than observed from outside by a neutral analyst.

Adolescent psychodynamic work must be developmentally flexible. Long silences, free association, and couch-based adult psychoanalysis are not automatically appropriate. Young people often need activity, clarity, family involvement, school liaison, humour, and explicit safety structures.

# 2. Core Concepts

## 2.1 Unconscious processes

Unconscious does not mean a mystical basement of forbidden content. In contemporary practice it includes processes outside awareness: automatic expectations, disowned feelings, procedural relational patterns, avoided memories, and motives that conflict with the person's self-image.

A student may consciously want to succeed yet repeatedly sabotage examinations because success feels disloyal to a struggling parent, exposes him to envy, or threatens an identity built around not caring. The counsellor does not announce this as fact. It becomes a tentative formulation tested against history, affect, repetition, and the young person's response.

## 2.2 Conflict and ambivalence

People can love and resent the same person, want independence and fear abandonment, desire success and dread visibility. Adolescence intensifies ambivalence because dependence and autonomy coexist.

Symptoms may compromise between wishes and prohibitions. School refusal can express fear, protect a depressed parent from loneliness, avoid humiliation, and punish controlling adults—all at once. A single-cause formulation may therefore be too thin.

## 2.3 Defences

Defences are psychological strategies that reduce awareness of painful feelings or conflict. They are adaptive in origin and costly when rigid.

- **Denial:** refusing to register reality.
- **Projection:** attributing one's disowned feeling to another.
- **Splitting:** experiencing self or others as all good or all bad.
- **Displacement:** redirecting feeling from a dangerous target to a safer one.
- **Intellectualisation:** discussing feeling abstractly to avoid experiencing it.
- **Humour:** transforming pain into shared perspective; it may be mature or avoidant depending on use.
- **Acting out:** communicating through action what cannot yet be thought or spoken.

The counsellor should describe defences compassionately. A defence is protection before it is pathology.

### 2.4 Transference and countertransference

Transference refers to the activation in therapy of expectations and feelings derived from earlier relationships. An adolescent may test whether the counsellor will become disgusted, intrusive, bored, rescuing, or abandoning.

Countertransference includes the counsellor's emotional responses, shaped both by the young person's interpersonal pull and by the counsellor's own history. It is data, not a command. Feeling bored may signal dissociation, hopelessness, practitioner fatigue, or an unspoken mismatch. Supervision prevents enactment.

### 2.5 Enactment and repetition

When patterns cannot be verbalised, they may be enacted. The counsellor is drawn into the role of critical parent, helpless child, rescuer, rival, or absent adult. Recognising enactment allows repair: “I notice I've started sounding like I'm cross-examining you. That may be exactly what happens at home. Let's pause.”

## 3. Object Relations and Internal Objects

An internal object is not a literal person inside the mind. It is an affect-laden representation of self in relation to another. A child repeatedly comforted develops an expectation that distress can be brought to someone and survived. A child repeatedly shamed may internalise a contemptuous other and a defective self.

These templates influence perception. Ambiguous teacher feedback is heard through the internal critic. A friend's silence activates an abandoning other. Therapy offers a new relational experience, but one relationship does not erase a template; repetition, reflection, family work, and real-world relationships all contribute.

Splitting may dominate under stress: one teacher is wonderful, another evil; mother is perfect until she sets a limit, then wholly cruel. Development involves integrating ambivalence—recognising that the same person can be loved, disappointing, safe, and limited.

## 4. Containment and Mentalization Links

Bion's concept of containment describes a caregiver receiving a child's unbearable state, tolerating and transforming it, and returning it in more manageable form. The parent who can hear panic without panicking or punishing helps the child learn that feeling can be thought about.

Psychodynamic adolescent practice often works through containment before interpretation. The counsellor remains present with rage, shame, or despair without retaliating, collapsing, or rushing to cheer up. Over time the young person internalises a more reflective relationship to their own states. This links psychodynamic work to attachment and mentalization approaches in Chapters 34 and 42.

## 5. Further Reading for This Chapter


- Freud, S. (1915). The unconscious. *Standard Edition, 14*.

- Winnicott, D. W. (1960). The theory of the parent-infant relationship. *International Journal of Psychoanalysis, 41*, 585–595.
- Klein, M. (1946). Notes on some schizoid mechanisms. *International Journal of Psychoanalysis, 27*, 99–110.
- Bion, W. R. (1962). *Learning from Experience*.
- Kohut, H. (1971). *The Analysis of the Self*.
- Lemma, A. (2016). *Introduction to the Practice of Psychoanalytic Psychotherapy* (2nd ed.).
- Midgley, N., Ensink, K., Lindqvist, K., Malberg, N., & Muller, N. (2017). *Mentalization-Based Treatment for Children*.


# 6. Psychodynamic Assessment and Formulation

A psychodynamic formulation is a structured hypothesis, not a pronouncement. It may include:

1. **Presenting pattern:** What repeats across settings and relationships?
2. **Developmental history:** Early care, losses, separations, trauma, illness, school transitions, and strengths.
3. **Internal world:** Representations of self and others; dominant affects; feared and wished-for relationships.
4. **Conflicts:** Autonomy versus loyalty, anger versus need, success versus belonging, sexuality versus shame, dependence versus contempt.
5. **Defences:** How painful states are avoided or communicated.
6. **Transference possibilities:** What role may the counsellor be assigned?
7. **Family and social context:** What present realities sustain the pattern?
8. **Capacity and risk:** Reflection, affect tolerance, support, self-harm, safeguarding, and ability to use therapy.

The formulation should be revisable. If every response confirms the theory—agreement means insight and disagreement means resistance—the theory has become unfalsifiable and clinically dangerous.

# 7. Technique with Adolescents

## 7.1 Frame and reliability

Time, place, confidentiality, cancellations, digital contact, and endings carry meaning. Reliability is treatment, especially for young people whose adults have been unpredictable. Changes should be explained and repaired.

## 7.2 Play, activity, and metaphor

Younger adolescents may think through drawing, music, games, stories, sport, or shared tasks. The counsellor follows meaning without forcing symbolic interpretation. A drawing of a locked house may be a house, a memory, a wish for privacy, or all three.

### 7.3 Clarification before interpretation

Ask what happened, what was felt, what was feared, and what was expected. Interpretation, when used, should be tentative, brief, close to experience, and timed when affect is present but manageable. “Perhaps when your tutor went quiet, it felt like the criticism you brace for at home” is preferable to a grand claim about the unconscious.

### 7.4 Working with defences

Do not strip defences faster than alternatives develop. Intellectualisation may allow a young person to attend sessions at all. Humour may preserve dignity. Denial may be temporarily necessary after loss. The counsellor names cost and function: “Joking helps you stay in the room; it also means no one knows when you're actually frightened.”

### 7.5 Rupture and repair

Missed appointments, silence, anger at the counsellor, and disappointment are material. Repair requires acknowledgement, curiosity, and sometimes apology. A counsellor who can survive anger without retaliation offers a powerful corrective experience.

## 8. Case Illustration: The Boy Who Made Teachers Give Up

**Presentation.** Yusuf, fifteen, is described as bright but sabotaging. He begins coursework enthusiastically, then stops near completion. Teachers initially invest heavily, become frustrated, and withdraw. Yusuf says he does not care.

**History and pattern.** His father, a high-achieving professional, left the family after repeated conflict and now offers intense praise during brief visits followed by long silence. Yusuf longs for his father's recognition and protects himself by deciding in advance that effort is pointless. Teachers repeat the cycle: special encouragement, disappointment, withdrawal. Yusuf interprets withdrawal as proof that adults value performance, not him.

**Transference.** In counselling, Yusuf tests the frame. He misses a session after a particularly open conversation. The counsellor feels irritated and then unusually inclined to send a long reassuring message. In supervision, this is understood as a pull to become the idealising rescuer who will later feel betrayed.

**Intervention.** The counsellor maintains boundaries, acknowledges the missed session without punishment, and explores sequence rather than motive. Yusuf can say, “When I tell people things, they get weird.” The counsellor responds, “Perhaps leaving first felt safer than waiting to see whether I would become less interested.” Yusuf is quiet, then says, “That's what Dad does—goes quiet.”

Work includes practical study structure, teacher consultation to provide steady rather than inflated feedback, and a family session in which his mother stops mediating all contact with his father. Yusuf writes but does not send several letters, distinguishing expression from action.

**Outcome.** Coursework completion improves unevenly. The more important shift is relational: Yusuf can tell a teacher, “I need feedback before the final week or I shut down,” instead of disappearing.

Psychodynamic formulation did not replace practical planning; it explained why planning collapsed at the point of anticipated judgement.

## 9. Evidence and Limitations

Psychodynamic therapies have evidence for several adult conditions and growing evidence for children and adolescents, including short-term psychodynamic psychotherapy for adolescent depression in some trials and service evaluations. Effects depend on condition, severity, treatment length, and comparison care. Evidence is less strong where immediate behavioural control, skills acquisition, family contingency change, or crisis stabilisation is the urgent need.

The approach's concepts vary in empirical status. Attachment and defence processes have research support; broad claims about unconscious causation are harder to test. Good practice uses psychodynamic thinking as formulation and relationship discipline, not as licence to override the young person's account.

Duration and access are concerns. Open-ended therapy may be unavailable or unnecessary. Brief psychodynamic models, mentalization-based treatment, and integration with family and school work improve applicability.

## 10. Cultural and Ethical Considerations

Psychodynamic theories emerged largely from European and North American clinical contexts. Concepts of self, family obligation, emotional expression, sexuality, and authority are culturally shaped. A counsellor must not interpret collectivist loyalty as enmeshment, religiously grounded moral commitments as mere defence, or justified mistrust of institutions as projection.

Interpretation is an exercise of power. The practitioner should invite correction, use tentative language, and avoid claiming privileged access to the young person's true motives. Safeguarding information must not be reframed as fantasy or transference. Where abuse, coercion, or discrimination is occurring, present reality takes precedence.

Confidentiality and parental involvement require developmental judgement. Parents may need guidance on patterns without receiving session content. Endings should be planned carefully where abandonment themes are central; abrupt service termination can repeat the very pattern under treatment.

## 11. Key Terms

**Key Terms at a Glance**

- **Defence** — A psychological strategy that reduces awareness of painful affect or conflict.
- **Transference** — Activation in therapy of relational expectations derived from earlier experience.
- **Countertransference** — The practitioner's emotional responses within the therapeutic relationship, used reflectively rather than acted upon.
- **Internal object** — An affect-laden mental representation of self in relation to a significant other.
- **Splitting** — Experiencing self or others in all-good and all-bad terms under stress.
- **Projective identification** — A process in which disowned states are evoked in another person, who may be pressured to enact them.
- **Containment** — Receiving and transforming unbearable affect so it can be returned in thinkable form.
- **Enactment** — Repetition of a relational pattern in action before it can be verbalised.

## 12. Chapter Summary

Psychodynamic and object relations approaches teach counsellors to listen for repetition, ambivalence, defence, and the relational meanings of behaviour. With adolescents, their value lies not in remote interpretation but in disciplined curiosity: What pattern is recurring? What feeling is being protected against? What role is the counsellor being invited to play? What new experience might make the pattern thinkable and therefore changeable?

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Explain unconscious process without using mystical language.
2. Distinguish projection, splitting, and displacement in school examples.
3. Why should defences be understood as protection before pathology?
4. How can countertransference be useful without becoming self-indulgent?
5. Write a tentative interpretation for a student who stops work near completion.
6. What makes a psychodynamic formulation falsifiable enough for ethical practice?
7. When should present safeguarding reality override exploration of meaning?

## 14. Practice Activities

**Practice and Application**

### Activity 47.1 — Pattern map

Map a fictional presenting problem across home, school, peers, and counselling. Identify repeated roles and affects.

### Activity 47.2 — Defence dialogue

Write two counsellor responses to intellectualisation: one confrontational and one function-respecting. Compare likely effects.

### Activity 47.3 — Rupture repair

Draft a repair statement after a counsellor arrives ten minutes late to meet a young person with abandonment fears.

## 15. Annotated Reading Pathways

**For clinical foundations.** Lemma provides a rigorous contemporary introduction without requiring allegiance to one school.

**For development.** Winnicott, Bion, and attachment research illuminate containment, play, and relational learning.

**For adolescents.** Read psychodynamic theory alongside mentalization, family systems, and trauma-informed practice so that internal formulation remains connected to present relationships and environments.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Psychodynamic and Object Relations Approaches** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the relational substrate of personality, transference, and developmental lines of interpretation. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture, relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete

categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Ines is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Ines's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Ines explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the

visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Ines chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Ines has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Ines can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.

- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Psychodynamic and Object Relations Approaches

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Psychodynamic and Object Relations Approaches** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Psychodynamic and**

**Object Relations Approaches** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

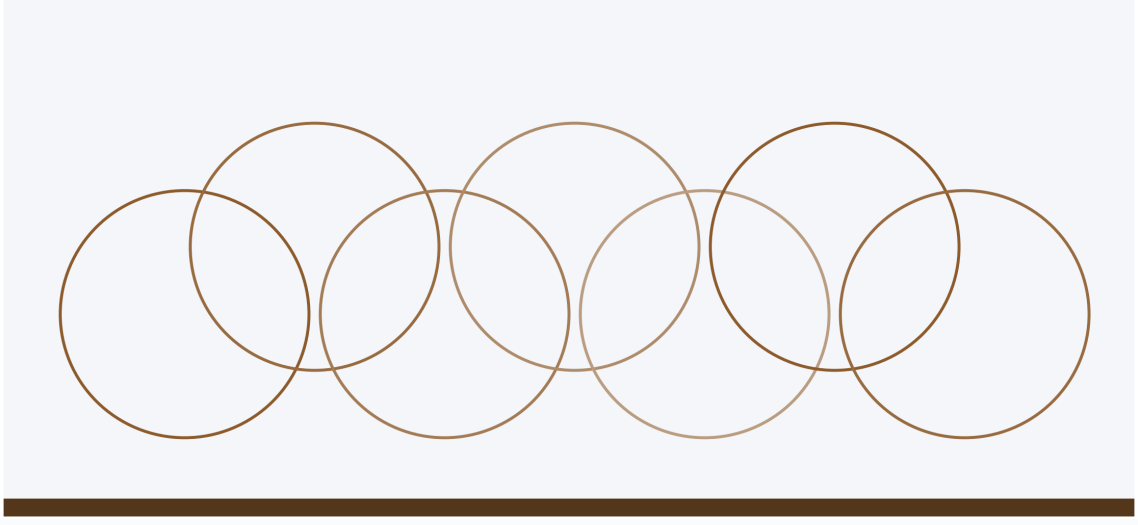
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Psychodynamic and Object Relations Approaches**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Psychodynamic and Object Relations Approaches to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 48

# Multicultural, Feminist, and Intersectional Theory

Power, voice, and privilege in the counseling relationship and in the theories that inform it.



## Learning Objectives

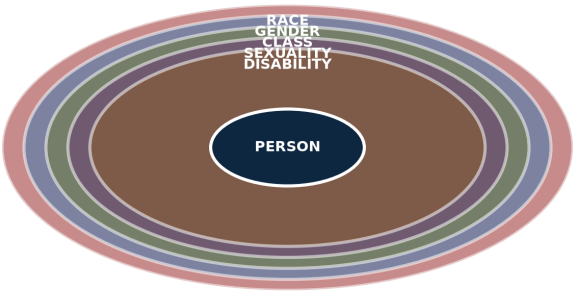
By the end of this chapter, the student should be able to:

- Explain why culture is not an optional variable added after theory but a context that shapes distress, identity, help seeking, and therapeutic power.
- Describe multicultural counselling competencies and cultural humility.
- Explain feminist theory's attention to power, gender, socialisation, and empowerment.
- Define intersectionality and apply it without reducing a young person to a list of categories.
- Use cultural formulation and broaching in assessment.
- Recognise cultural bias in diagnosis, testing, school discipline, and theory selection.
- Practise anti-oppressive, culturally sustaining counselling with children, adolescents, families, and institutions.

## Chapter Overview

Every counselling theory carries assumptions about what a person is, what a family should look like, what emotions mean, what counts as maturity, and who has authority to define a problem. Multicultural, feminist, and intersectional theories make those assumptions visible. They ask not only “What is happening inside this young person?” but also “What has happened around them, what meanings are available to them, whose knowledge is being trusted, and what power does the counsellor hold?”

**Intersecting Systems of Privilege and Oppression**




*Oppression is not additive but multiplicative: each system alters the meaning of the others.*

**Figure 48.38** Intersectionality: overlapping social identities and systems of power shape experience in ways that cannot be understood by adding categories separately.

## 1. Culture as Context, Not Decoration

Culture includes ethnicity, race, language, migration, nationality, religion or non-religious worldview where relevant to the client, class, disability culture, gender norms, sexuality, family forms, region, and

institutional cultures such as school or care systems. This textbook avoids faith-based structures, but culturally responsive practice still requires recognising when a client identifies religious or spiritual commitments as personally important, without the counsellor importing religious content or practice.

Cultural competence is not mastery of a catalogue of group traits. That approach produces stereotypes. Contemporary practice emphasises **cultural humility**: lifelong self-critique, recognition of power imbalance, openness to learning from the client, and institutional accountability.

The counsellor asks: What should I understand about your family, community, language, and experiences for this work to be useful? What have services misunderstood before? Who should be involved? What would respect look like here?

## 2. Multicultural Counselling Theory

Multicultural counselling developed as scholars and practitioners challenged the assumption that theories derived from Western, educated, individualist populations were universal. Sue and colleagues articulated competencies in awareness, knowledge, and skills; later frameworks added action and advocacy.

Key concerns include:

- **Worldview:** individual versus collective decision-making, direct versus indirect communication, time orientation, emotional expression, and concepts of illness and healing.
- **Identity development:** how young people understand ethnicity, race, culture, migration, language, and belonging across contexts.
- **Acculturation and enculturation:** adaptation to a dominant culture and connection to heritage culture, neither inherently healthy or unhealthy.
- **Discrimination and racial trauma:** repeated invalidation, surveillance, exclusion, and threat can shape stress physiology, identity, school trust, and help seeking.
- **Language access:** assessment through an interpreter requires trained interpretation, confidentiality, and awareness that family members—especially children—should not interpret sensitive material.

## 3. Feminist Theory

Feminist counselling emerged from recognition that psychological distress cannot be separated from gendered power, violence, labour, caregiving, body norms, and social expectations. Its principles include egalitarian relationship, valuing lived experience, demystifying therapy, empowerment, social change, and attention to intersectional identities.

With adolescents, feminist practice asks how gender norms regulate behaviour. Girls may be rewarded for compliance and punished for anger; boys may be shamed for vulnerability and rewarded for dominance; non-binary and transgender young people may face invalidation or danger. But feminist analysis does not assume all girls are passive or all boys are emotionally illiterate. It examines specific social messages and their costs.

Feminist practice also notices adult gender patterns in the room: mothers blamed for family distress, fathers treated as peripheral, female staff expected to absorb emotional labour, or male counsellors presumed unsafe or authoritative. The goal is not to reverse blame but to redistribute responsibility and voice.

## 4. Intersectionality

Intersectionality, developed in Black feminist legal scholarship by Kimberlé Crenshaw, shows how systems such as racism, sexism, class inequality, ableism, homophobia, transphobia, and age hierarchy interact. A disabled Black girl in care, a working-class migrant boy, and an affluent white LGBTQ+ student may all experience "school anxiety", but the mechanisms, risks, resources, and institutional responses differ.

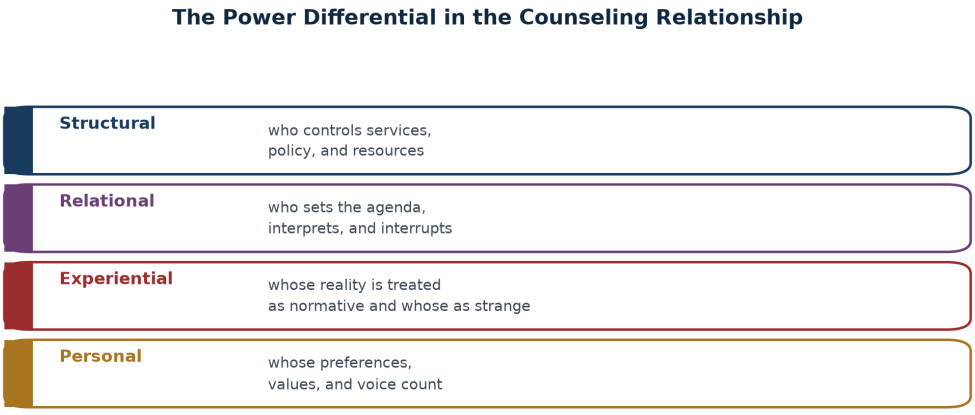



**Figure 48.39** Power differentials in counselling: how professional, adult, institutional, linguistic, and economic power affect voice and choice.

Intersectional practice avoids additive checklists. It asks which identities are salient to the young person, which systems are active in this situation, and where the counsellor's own social position affects perception. It also recognises privilege within marginalised groups and marginalisation within privileged groups.

## 5. Minority Stress and Structural Formulation

Minority stress theory distinguishes distal stressors—discrimination, rejection, victimisation—from proximal processes such as internalised stigma, concealment, and expectation of rejection. Structural formulation extends case conceptualisation to housing, income, transport, school policy, policing, immigration status, health access, language, and disability access.

This matters because a counsellor can otherwise teach coping for conditions that should be changed. Breathing exercises do not remedy racist discipline. Self-esteem work does not replace accessible toilets, interpreters, safe transport, or anti-bullying enforcement. Culturally responsive counselling therefore includes advocacy and consultation.

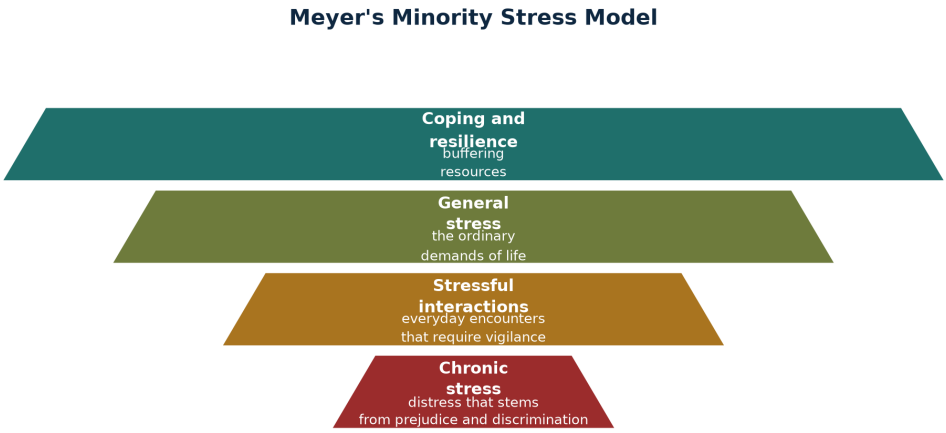



**Figure 48.40** Minority stress processes linking social stigma and discrimination to vigilance, concealment, internalised stigma, and health outcomes.

## 6. Further Reading for This Chapter

- Sue, D. W., Sue, D., Neville, H. A., & Smith, L. (2022). *Counseling the Culturally Diverse* (9th ed.).
- Crenshaw, K. (1989). Demarginalizing the intersection of race and sex. *University of Chicago Legal Forum, 1989*(1), 139–167.
- Hook, J. N., Davis, D. E., Owen, J., Worthington, E. L., & Utsey, S. O. (2013). Cultural humility: Measuring openness to culturally diverse clients. *Journal of Counseling Psychology, 60*(3), 353–366.
- Ratts, M. J., Singh, A. A., Nassar-McMillan, S., Butler, S. K., & McCullough, J. R. (2016). Multicultural and Social Justice Counseling Competencies. *Journal of Multicultural Counseling and Development, 44*(1), 28–48.
- Worell, J., & Remer, P. (2003). *Feminist Perspectives in Therapy*.
- Meyer, I. H. (2003). Prejudice, social stress, and mental health in lesbian, gay, and bisexual populations. *Psychological Bulletin, 129*(5), 674–697.

## 7. Broaching and Cultural Formulation

Broaching is the counsellor's intentional invitation to discuss culture, identity, power, and difference. Silence does not create neutrality; it often tells the young person that these topics are unsafe or irrelevant.

A developmentally appropriate broach might be: “People's experiences at school can be affected by race, language, disability, gender, family income, or being new to a place. I don't want to assume which, if any, matter for you. Are any of those part of what you're dealing with?”

Cultural formulation explores:

- the young person's and family's explanation of the problem;
- salient identities and communities;

- migration, language, and intergenerational differences;
- discrimination and institutional experiences;
- family roles, authority, privacy, and decision-making;
- strengths, cultural resources, and trusted helpers;
- differences between client and counsellor;
- barriers to services and previous experiences of help.

The DSM-5 Cultural Formulation Interview provides useful prompts, though it should be adapted for adolescents and not treated as an interrogation.

## 8. Assessment, Diagnosis, and Testing

Cultural bias can enter through language, norms, item content, examiner behaviour, and interpretation. A quiet adolescent may be respectful, frightened, dissociating, neurodivergent, unfamiliar with the language of assessment, or uninterested. A parent who challenges a professional may be advocating within a system that has previously ignored them.

Before diagnosing, ask whether behaviour is developmentally expected, culturally congruent, situationally adaptive, or produced by unmet access needs. Use multiple informants and settings, validated translations, interpreters, and dynamic assessment where appropriate. Document uncertainty.

School data require disaggregation and scepticism. If one group receives more exclusions, restraint, lower grades for similar work, or fewer referrals to support, individual counselling cannot be the sole response. The institution becomes part of the formulation.

## 9. Case Illustration: “Defiant” in Two Languages

**Presentation.** Samira, fourteen, recently arrived in a new country, is referred for refusing to speak in class and arguing with her mother at home. School staff suggest selective mutism and oppositional behaviour.

**Cultural formulation.** Samira speaks three languages but is still acquiring academic English. In class she fears pronunciation errors after students imitated her accent. At home she interprets for her mother at appointments, giving her adult responsibility but little authority. Her mother, once a pharmacist, works nights cleaning and feels grief and status loss. Arguments occur when Samira refuses further interpreting for complex medical letters.

**Intersectional reading.** Gender, migration, language, class change, age, and parentification interact. Samira is simultaneously privileged by adults as competent and burdened by that competence. Her silence protects dignity at school; her anger at home protests an unsustainable role.

**Intervention.** The school provides language support, private ways to answer, anti-bullying action, and a bilingual staff contact. The service uses professional interpreters and translated letters so Samira is not the family's administrative bridge. Counselling validates grief and status changes, helps mother and daughter negotiate interpreting boundaries, and builds Samira's confidence through small speaking successes. The counsellor does not diagnose defiance from behaviour that had communicative and structural logic.

## 10. Feminist and Anti-Oppressive Practice in Sessions

Practical commitments include:

- explain counsellor power, records, confidentiality, and choices in accessible language;
- invite correction and treat disagreement as information;
- avoid mother blaming and include fathers, kin, and chosen supports where safe;
- notice whose time, body, privacy, and labour are expected to adjust;
- make goals with the young person, not merely for institutional convenience;
- use interpreters and accessible formats;
- challenge discriminatory policies through consultation and governance;
- evaluate outcomes by client-defined wellbeing and participation, not compliance alone.

Self-reflection must lead somewhere. A counsellor who recognises bias but continues using the same referral thresholds, language, and punitive partnerships has insight without action.

## 11. Evidence and Limitations

Evidence supports culturally adapted interventions for some populations and outcomes, though adaptation is heterogeneous. Cultural humility is associated with stronger alliance and client perceptions of counsellor competence. Feminist and empowerment-oriented approaches have supportive but variable evidence depending on population and outcome.

No competency framework can guarantee responsive practice. Training can become performative if institutions do not change staffing, language access, data monitoring, discipline patterns, and decision-making power. Intersectionality can also be diluted into a slogan. Its test is whether formulation and action change.

## 12. Key Terms

**Key Terms at a Glance**

- **Cultural humility** — Lifelong self-critique, openness, and accountability in cross-cultural relationships.
- **Broaching** — Intentionally inviting discussion of culture, identity, difference, and power.
- **Intersectionality** — Analysis of interacting social identities and systems of power.
- **Minority stress** — Stress arising from stigma, discrimination, concealment, and expectation of rejection.
- **Structural formulation** — Case conceptualisation that includes material and institutional conditions.
- **Parentification** — A child assuming adult practical or emotional responsibilities.
- **Anti-oppressive practice** — Counselling that identifies and acts against power relations producing harm.

## 13. Chapter Summary

Multicultural, feminist, and intersectional theories change the counsellor's questions. Distress is still personal, but never only personal. Identity, language, power, discrimination, family roles, and institutional design shape both suffering and recovery. Ethical practice therefore combines humility in the room with action beyond it.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Why is cultural competence as a trait checklist inadequate?
2. Write two appropriate broaching statements for adolescents.
3. Apply intersectionality to school exclusion without making an additive list.
4. How does feminist theory alter mother blaming in family work?
5. Distinguish cultural formulation from stereotyping.
6. Identify three points where assessment bias can enter.
7. When does coping work become ethically insufficient without advocacy?

## 15. Practice Activities

**Practice and Application**

### Activity 48.1 — Assumption audit

Take a familiar theory from this book and list its assumptions about selfhood, family, emotion, communication, and maturity.

### Activity 48.2 — Structural formulation

Rewrite a fictional case formulation twice: once individual-only and once including language, income, school policy, disability access, and discrimination.

### Activity 48.3 — Institutional data

Design three questions a school counselling service should ask of its referral, exclusion, and outcome data to detect inequity.

## 16. Annotated Reading Pathways

**For competencies.** Sue and colleagues and the Multicultural and Social Justice Counseling Competencies provide frameworks, but read them alongside cultural humility research.

**For intersectionality.** Begin with Crenshaw and Black feminist scholarship rather than secondary summaries alone.

**For practice.** Combine cultural formulation with school consultation, interpreter-mediated therapy, disability justice, and anti-racist organisational change.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Multicultural, Feminist, and Intersectional Theory** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is power, voice, and privilege in the counseling relationship and in the theories that inform it. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture, relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete

categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Kofi is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Kofi's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Kofi explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the

visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Kofi chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Kofi has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Kofi can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.

- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Multicultural, Feminist, and Intersectional Theory

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Multicultural, Feminist, and Intersectional Theory** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Multicultural,**

**Feminist, and Intersectional Theory** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

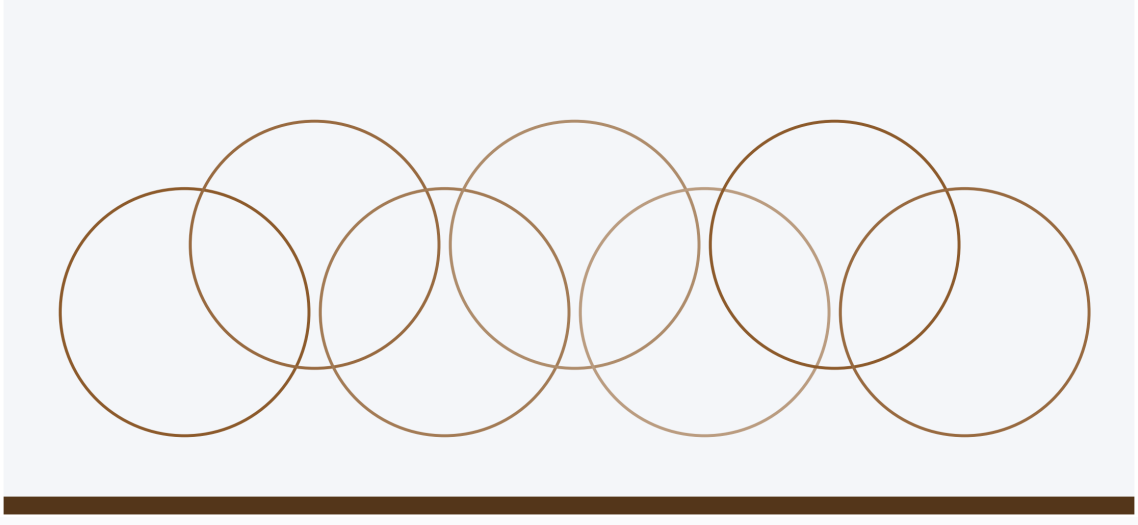
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Multicultural, Feminist, and Intersectional Theory**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Multicultural, Feminist, and Intersectional Theory to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 49

# Affirmative Practice with LGBTQ+ Youth

Gender-responsive and affirming practice, the role of the therapeutic alliance, and the harms of conversion practices.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Use respectful, developmentally appropriate language about sexual orientation, gender identity, gender expression, and sex characteristics.
- Explain minority stress, family acceptance, school safety, and identity development.
- Distinguish affirmation from pressure to label, disclose, or transition in any direction.
- Conduct risk, safety, family, and school assessments without making identity itself the pathology.
- Develop confidentiality and disclosure plans with LGBTQ+ adolescents.
- Respond ethically to family rejection, bullying, conversion efforts, and institutional discrimination.
- Evaluate evidence and legal variation while maintaining professional standards of dignity and safety.

## Chapter Overview

Affirmative practice begins from a clear professional position: lesbian, gay, bisexual, transgender, queer, questioning, intersex, asexual, and other sexual and gender minority identities are variations of human experience, not mental disorders. Distress commonly arises from stigma, rejection, bullying, concealment, unsafe environments, and barriers to care. The counsellor's task is not to steer a young person toward or away from an identity, but to support safety, exploration, self-understanding, family communication where safe, and access to appropriate resources.

Adolescents may use labels, reject labels, change labels, or hold uncertainty. Developmentally responsive practice follows the young person's language while keeping conversation open. A label can provide community and coherence; it can also feel constraining. Neither certainty nor questioning requires correction.

## 1. Language and Concepts

**Sexual orientation** concerns patterns of romantic, emotional, and/or sexual attraction. **Gender identity** is a person's internal sense of gender. **Gender expression** includes clothing, voice, mannerisms, name, and presentation. **Sex assigned at birth** is a classification made from observed characteristics and is not identical to gender identity. **Intersex** refers to variations in sex characteristics. **Cisgender** describes people whose gender identity aligns with sex assigned at birth; **transgender** describes those whose identity differs. **Non-binary** is an umbrella term for identities not exclusively man or woman.

Names and pronouns matter. Ask privately what name and pronouns the young person uses in different settings. A name used with friends may not be safe at home or school. Record systems can unintentionally disclose information; practitioners must know what appears on letters, registers, appointment texts, and shared records.

Sexual orientation and gender identity are not phases to be diagnosed by adults. Exploration is normal, and so is early certainty. Some young people know from childhood; others understand themselves later. Neither timeline is more authentic.

## 2. Minority Stress and Resilience

Minority stress includes bullying, slurs, exclusion, family rejection, discriminatory policies, online harassment, expectation of rejection, concealment, and internalised stigma. These stressors help explain disparities in depression, anxiety, self-harm, suicide risk, substance use, homelessness, and school disengagement. Disparity is not destiny, and group statistics must never be used to presume an individual's experience.

Resilience is equally important. LGBTQ+ youth build chosen families, friendships, creative communities, online support, humour, activism, and pride. Affirmative counselling identifies resources without romanticising adversity. A young person should not have to become exceptionally resilient because adults refuse to make environments safe.

Family acceptance is a major protective factor. Research associated with the Family Acceptance Project links specific accepting behaviours—listening, advocating, welcoming friends, supporting expression, connecting with resources—to better health outcomes. Rejection behaviours—blocking access to LGBTQ+ friends, blaming the young person for bullying, forcing concealment, or using identity as an insult—are associated with poorer outcomes.

School belonging matters. Inclusive curricula, anti-bullying enforcement, gender-neutral facilities where available, supportive staff, student clubs where lawful and safe, and protection from forced outing can alter daily stress more powerfully than weekly counselling alone.

## 3. Identity Development without a Script

Older stage models proposed linear movement from confusion to pride and synthesis. Contemporary practice treats identity development as fluid, contextual, and sometimes non-linear. A young person may be out to friends but not family, use one name online and another at school, or identify differently across time.

Exploration may involve clothing, names, pronouns, dating, community, media, language, and imagination. The counsellor should distinguish exploration from risk behaviour. Trying a new name is not equivalent to unsafe sexual activity; attending a pride event is not pathology; asking questions about bodies requires accurate information and safeguarding, not shame.

Assessment asks:

- What words, if any, feel right now?
- Where is it safe or unsafe to be known?
- Who knows, who does not, and who controls disclosure?
- What support exists?
- What stress is linked to identity, and what is unrelated?
- What does the young person want from counselling?

## 4. Risk and Safety Assessment

Risk assessment should be specific and non-sensational. Ask about suicidal thoughts, self-harm, bullying, sexual exploitation, violence, running away, homelessness, substance use, eating concerns, and access to

support. Do not assume identity causes risk; identify mechanisms and contexts.

Safety planning includes trusted adults, safe routes, online privacy, emergency contacts, school staff, community organisations, and what to do if outed. Digital safety requires attention to shared devices, location tracking, screenshots, cloud backups, and messages that may be discovered.

Conversion efforts—practices intended to change sexual orientation or gender identity—lack credible evidence of effectiveness and are associated with harm. They are prohibited or restricted in many jurisdictions and opposed by major professional bodies. Counsellors must not provide, refer to, or disguise such practices as neutral exploration. Ethical exploration supports the young person's self-understanding without a predetermined identity outcome.

## 5. Further Reading for This Chapter


- American Psychological Association. (2021). Guidelines for psychological practice with sexual minority persons. *American Psychologist, 76*(7), 974–994.
- American Psychological Association. (2015). Guidelines for psychological practice with transgender and gender nonconforming people. *American Psychologist, 70*(9), 832–864.
- Ryan, C., Huebner, D., Diaz, R. M., & Sanchez, J. (2009). Family rejection as a predictor of negative health outcomes in white and Latino lesbian, gay, and bisexual young adults. *Pediatrics, 123*(1), 346–352.
- Meyer, I. H. (2003). Prejudice, social stress, and mental health in lesbian, gay, and bisexual populations. *Psychological Bulletin, 129*(5), 674–697.
- Russell, S. T., & Fish, J. N. (2016). Mental health in lesbian, gay, bisexual, and transgender youth. *Annual Review of Clinical Psychology, 12*, 465–487.
- GLSEN. (2022). *The 2021 National School Climate Survey*.


## 6. Confidentiality, Names, and Disclosure

Confidentiality can be a safety intervention. Before counselling begins, explain limits in plain language and identify exactly what may be shared with parents, school staff, health providers, and safeguarding services. Distinguish safety information from identity information. A parent may need to know that a young person is self-harming without being told every detail of sexuality or gender exploration, subject to law and risk.

Disclosure planning asks:

1. What might be gained and risked by telling each person?
2. What is the likely response, based on evidence rather than hope alone?
3. What timing, setting, and medium are safest?
4. Who can be present or nearby?
5. What housing, financial, educational, and emotional supports exist if the response is poor?
6. What boundaries will the young person set about questions and further disclosure?

The counsellor does not pressure coming out as a marker of therapeutic progress. Concealment can be costly, but it can also be protective. The ethical goal is controlled, informed choice where possible.

Records require special care. Use chosen name and pronouns in the therapeutic record where systems allow, note context-specific names, and avoid unnecessary recording of sensitive details. Appointment reminders and school registers can out a student. Practitioners should advocate for record systems that protect privacy and dignity.

## 7. Family Work

Family responses range from celebration to grief, confusion, anger, fear, or rejection. Initial shock is not always final rejection. Parents may need space to ask questions without making the young person responsible for educating or comforting them.

Family sessions should not become debates about the legitimacy of the young person's identity. Ground rules can include respectful names and pronouns, no insults, no threats, no forced disclosure to extended family, and pauses when conversation escalates. Separate parent sessions may help adults process fears, obtain accurate information, and identify concrete accepting behaviours.

Concrete acceptance includes listening without immediate correction, defending the young person against bullying, welcoming friends, supporting safe expression at home, using the chosen name where agreed, and helping access supportive communities. Where family behaviour creates danger, safeguarding and alternative support take precedence over preserving a joint format.

Cultural and religious contexts require respectful complexity. This textbook does not use faith-based structures, and counsellors should not impose theological arguments. They can, however, help families distinguish their values and fears from immediate safety behaviours: even where beliefs differ, violence, expulsion, humiliation, and withdrawal of care cause harm. Referral to culturally congruent family resources may help when chosen by the family.

## 8. School Practice

School counsellors can assess bullying patterns, staff responses, facilities, dress codes, sports access, curriculum inclusion, and confidentiality in registers and communications. Interventions may include safety routes, trusted staff, timetable adjustments, anti-bullying action, peer bystander work, and advocacy.

Do not make the targeted student responsible for educating the whole school or repeatedly recounting harm. Incident systems should record patterns, locations, and adult responses. Restorative approaches are inappropriate where they force contact with an unsafe perpetrator or minimise identity-based harassment.

Clubs and peer groups can be protective where they are permitted and safely facilitated. If a club would expose students to danger, alternatives may include off-site youth services, moderated online communities with safeguarding, library resources, and individual mentoring.

## 9. Health and Specialist Referral

Counsellors should know local pathways for sexual health, HIV prevention and care, substance use, eating disorders, trauma, housing, and gender-related health care. Gender care for minors is regulated differently across jurisdictions and may involve specialist multidisciplinary assessment. The counsellor's role is supportive, non-directive regarding identity outcome, accurate about local process, and attentive to distress,

family communication, school safety, and co-occurring needs while referrals proceed.

Avoid acting as gatekeeper beyond professional competence. Do not require a young person to prove identity through stereotypes. Do not delay mental health support while specialist assessment is pending. Coordinate with consent and document who holds responsibility for each part of care.

## 10. Case Illustration: Two Names, One Safety Plan

**Presentation.** Alex, fifteen, uses the name Alex and they/them pronouns with friends. At home and school, records use a different name. Alex has begun skipping lunch to avoid a changing-room corridor where students shout slurs. Their mother has requested counselling because Alex is “secretive and angry”.

**Assessment.** Alex is not depressed about being non-binary; they are exhausted by vigilance and by correcting adults. Home response is uncertain. An older sibling is supportive. Alex fears a school letter will reveal the chosen name.

**Intervention.** The counsellor first addresses administrative safety: appointment messages go to Alex's phone, not a shared family account; the counsellor checks what name appears on school sign-in sheets. Together they identify a trusted teacher and an alternative lunch space that does not isolate Alex from friends.

Disclosure is planned in stages. Alex tells their mother with the sibling nearby at home, after preparing answers and an exit plan to the sibling's room. The counsellor holds a separate parent session focused on immediate behaviours: listen, use Alex at home if Alex confirms, do not inform extended family, and challenge corridor bullying narratives that blame Alex's expression.

School action targets location and supervision rather than requiring Alex to report every incident publicly. Over time, Alex eats with friends again. Family conversations remain imperfect, but the mother practises the chosen name and stops threatening to remove internet access as a response to identity. The outcome is not framed as Alex becoming more resilient to harassment; adults reduce exposure and increase support.

## 11. Evidence and Limitations

Research consistently associates family acceptance, school belonging, supportive policies, and access to affirming communities with better mental health among LGBTQ+ youth. Interventions that reduce bullying and rejection operate at plausible causal pathways, though many studies are observational and contexts vary.

Evidence for specific counselling models designed solely for LGBTQ+ adolescents is still developing. Affirmative adaptations of cognitive-behavioural and family interventions show promise. Practitioners should monitor outcomes rather than assume affirmation as an attitude guarantees effective technique.

Legal and policy environments vary and can change. Students must learn local law on consent, confidentiality, safeguarding, education, health referrals, and discrimination, while recognising that professional ethics may require advocacy where law or institutional practice creates avoidable harm.

## 12. Key Terms

**Key Terms at a Glance**

- **Affirmative practice** — Care that validates sexual and gender diversity and addresses stigma-related stress without pathologising identity.
- **Chosen name** — The name a young person uses for themselves, which may vary by safety context.
- **Outing** — Disclosing someone's sexual orientation or gender identity without consent.
- **Family acceptance** — Concrete behaviours that communicate support, protection, and belonging.
- **Conversion efforts** — Practices intended to change sexual orientation or gender identity; unsupported and associated with harm.
- **Chosen family** — Supportive relationships formed beyond biological or legal family.
- **Minority stress** — Stress produced by stigma, discrimination, vigilance, concealment, and internalised negativity.

## 13. Chapter Summary

Affirmative practice is neither neutral about harm nor prescriptive about identity. It protects confidentiality, follows the young person's language, reduces environmental danger, builds family acceptance where possible, and treats resilience as something communities can support rather than demand. The counsellor's credibility rests on small administrative details as much as large statements: the right name, a safe message, a private record, a teacher who intervenes, and a door that does not have to be explained every day.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish sexual orientation, gender identity, gender expression, and sex assigned at birth.
2. Why is coming out not automatically a therapeutic goal?
3. List five concrete family acceptance behaviours.
4. How can ordinary record systems create risk?
5. Why are conversion efforts ethically unacceptable?
6. Design a school safety response that does not burden the targeted student.
7. What should a counsellor do while a young person awaits specialist gender-related assessment?

## 15. Practice Activities

**Practice and Application**

### Activity 49.1 — Context map

Create a fictional adolescent's map of home, school, friends, online spaces, and services. Record name, pronouns, outness, support, and risk for each context.

### Activity 49.2 — Disclosure plan

Write a staged disclosure plan that includes timing, support person, likely responses, boundaries, and contingency housing or transport.

### Activity 49.3 — Administrative audit

Audit a fictional counselling service's forms, reminders, waiting-room procedures, and school reports for accidental outing risks.

## 16. Annotated Reading Pathways

**For guidelines.** APA guidelines provide detailed competencies for sexual minority and transgender and gender nonconforming practice.

**For family work.** Family Acceptance Project materials translate research into specific caregiver behaviours.

**For schools.** GLSEN and comparable school climate research show how policy, staff intervention, facilities, and peer belonging shape daily safety.

**For local practice.** Learn current regional law and reputable youth, housing, sexual health, and crisis services before referrals are needed.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Affirmative Practice with LGBTQ+ Youth** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is gender-responsive and affirming practice, the role of the therapeutic alliance, and the harms of conversion practices. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture, relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements;

others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Amara is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Amara's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Amara explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through

repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Amara chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Amara has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Amara can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Affirmative Practice with LGBTQ+ Youth

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Affirmative Practice with LGBTQ+ Youth** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Affirmative Practice with LGBTQ+ Youth** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

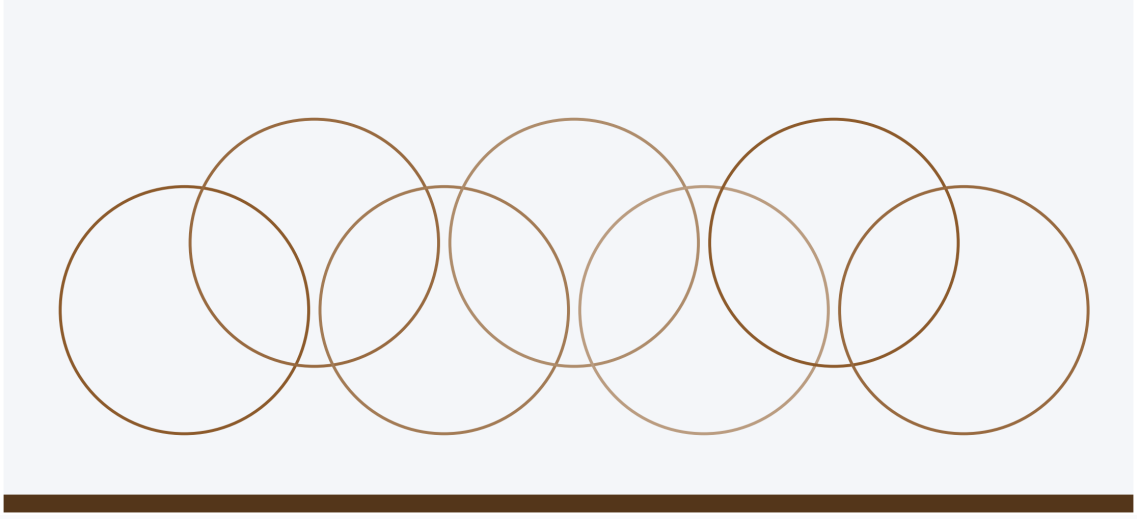
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Affirmative Practice with LGBTQ+ Youth**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Affirmative Practice with LGBTQ+ Youth to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 50

# Resilience, Ecological Systems, and Strengths-Based Practice

Bronfenbrenner, Masten's resilience research, and the systematic mobilisation of protective factors.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Define resilience as a dynamic process rather than a personality trait.
- Describe Bronfenbrenner's ecological systems and their relevance to case formulation.
- Identify individual, relational, school, community, and cultural protective processes.
- Explain risk, protective, promotive, and compensatory factors.
- Use strengths-based assessment without denying adversity or transferring responsibility to the young person.
- Design multi-level interventions for individuals, families, schools, and communities.
- Evaluate evidence, cultural considerations, and ethical risks.

## Chapter Overview

Resilience is often misunderstood as toughness: the child who “bounces back”, copes without complaint, and asks little from adults. That definition is both scientifically wrong and ethically dangerous. Resilience is better understood as the capacity of a developing system—child, family, school, community—to adapt successfully under adversity. It depends on relationships, resources, opportunities, biology, culture, and timing. A young person may function well academically while struggling emotionally, cope at home but not at school, or deteriorate when a single key support is removed.

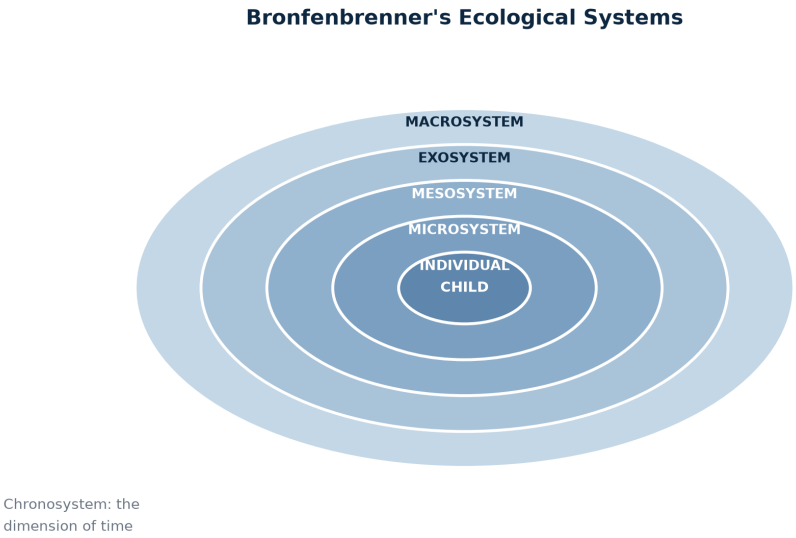



**Figure 50.41** Ecological systems: the developing young person nested within family, school, peer, community, cultural, policy, and time contexts.

## 1. Ecological Systems Theory

Bronfenbrenner's model places development within interacting contexts.

**Microsystem:** immediate settings such as family, classroom, peer group, team, and online community.

**Mesosystem:** connections among microsystems—for example, whether school and family communicate respectfully, or whether a youth service and health clinic coordinate.

**Exosystem:** settings that affect the young person indirectly, including parental employment, school funding, transport, housing services, and media systems.

**Macrosystem:** cultural values, laws, economic structures, discrimination, and beliefs about childhood, disability, gender, and success.

**Chronosystem:** time and transitions—developmental stage, historical events, migration, bereavement, policy change, and the duration of adversity.

Ecological formulation prevents the counsellor from locating every problem in the nearest person. A student's inattention may involve sleep, classroom noise, hunger, parental shift work, bus routes, medication timing, peer conflict, and an undiagnosed learning difficulty. Each level suggests different interventions and different responsible actors.

## 2. Risk and Protection

A **risk factor** increases probability of poor outcome. A **protective factor** reduces the effect of risk. A **promotive factor** supports positive outcomes across levels of risk. These are probabilistic, not destiny.

Risk accumulates. One stressor may be manageable; poverty, unstable housing, parental illness, school exclusion, discrimination, and bereavement together can overwhelm systems. Timing matters: the same event has different implications at age five, twelve, or seventeen. Duration and controllability matter too.

Protection often comes from ordinary systems—what Ann Masten calls “ordinary magic”. These include:

- at least one stable, committed adult relationship;
- effective caregiving and family organisation;
- cognitive, language, and self-regulation skills;
- school belonging and opportunities for mastery;
- friendships and prosocial peer groups;
- cultural identity, community participation, and meaningful roles;
- safe housing, nutrition, sleep, health care, and transport;
- services that coordinate rather than fragment care.

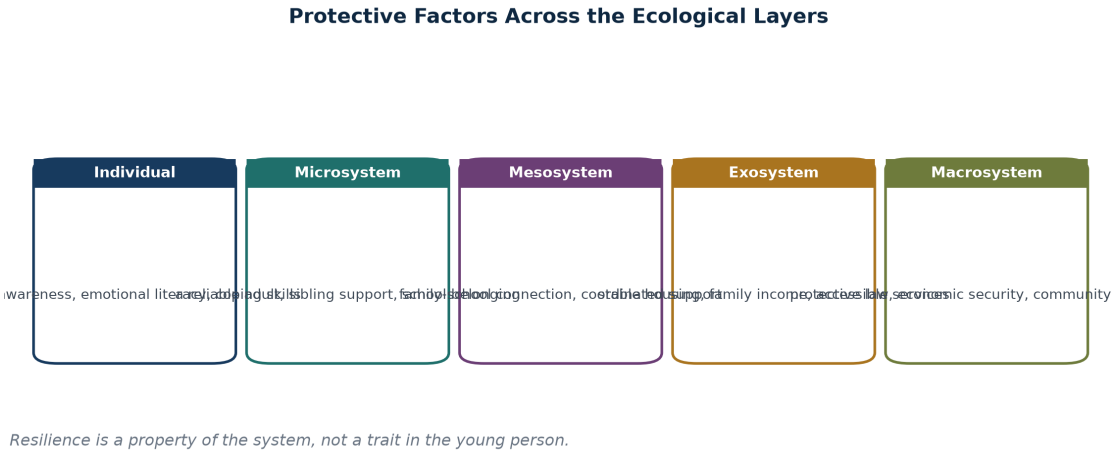



**Figure 50.42** A resilience map showing risk load, protective processes, and intervention points across individual, relational, school, and community systems.

## 3. Resilience as Process

Resilience is not a permanent state. A young person may be resilient in relation to academic performance after bereavement but not in relation to trust. Another may manage chronic illness with remarkable organisation while finding peer rejection destabilising.

Assessment should therefore specify:

1. Resilient in which domain?
2. Under what adversity?
3. Over what period?
4. Supported by which processes?
5. At what hidden cost?

The last question matters. A young person who cares for siblings, suppresses distress, and achieves highly may be praised as resilient while experiencing exhaustion, loneliness, or delayed developmental opportunities. Adults should not use resilience language to justify withholding help.

## 4. Strengths-Based Practice

Strengths-based practice begins by identifying capacities, interests, relationships, cultural resources, previous coping, values, and hopes. It asks what has survived, what matters, who helps, and what a good-enough future would look like.

It is not positive thinking. It does not require reframing abuse as growth, poverty as character-building, or discrimination as an opportunity. Strengths are resources for action, not excuses for institutions.

Good strengths assessment is specific. “She is resilient” is weak. “She maintains friendships, notices when her younger brother is overwhelmed, uses music to regulate, attends regularly when transport is available, and trusts one teacher” is usable. Each strength suggests an intervention pathway.

## 5. Developmental Cascades

Small changes can cascade. Improved sleep improves attention; attention improves teacher feedback; feedback improves academic efficacy; efficacy increases attendance; attendance expands friendships. Negative cascades also occur: exclusion reduces learning, peer contact, family income through parental work absence, and trust in institutions.

Ecological practice searches for high-leverage points where modest, feasible change can start a positive cascade. Sometimes the best counselling intervention is a bus pass, a timetable adjustment, a hearing assessment, a parent work-schedule letter, or repair of one teacher relationship.

## 6. Further Reading for This Chapter

- Bronfenbrenner, U. (1979). *The Ecology of Human Development*.
- Masten, A. S. (2001). Ordinary magic: Resilience processes in development. *American Psychologist, 56*(3), 227–238.
- Masten, A. S. (2014). *Ordinary Magic: Resilience in Development*.
- Ungar, M. (2011). The social ecology of resilience. *American Journal of Orthopsychiatry, 81*(1), 1–17.
- Rutter, M. (2012). Resilience as a dynamic concept. *Development and Psychopathology, 24*(2), 335–344.
- Saleebey, D. (Ed.). (2013). *The Strengths Perspective in Social Work Practice* (6th ed.).

## 7. Ecological Assessment

An ecological assessment can be organised as a visual map with the young person at the centre and rings for family, peers, school, services, community, culture, policy, and time.

For each ring, ask:

- What pressures operate here?
- What resources operate here?
- Who has power?
- What relationship connects this ring to the others?
- What can be changed, by whom, and how quickly?
- What must be endured safely while longer change is pursued?

The map should include online environments and informal supports: extended kin, neighbours, coaches, faith-neutral community groups where relevant, youth workers, cultural organisations, libraries, and safe public spaces.

Assessment also examines service ecology. Multiple professionals may each hold a partial plan. The young person repeats their story, receives contradictory advice, and attends appointments that conflict with school. Coordination is a clinical intervention.

# 8. Multi-Level Intervention

## Individual level

Skills, therapy, health care, sleep, learning support, assistive technology, and opportunities for mastery.

## Relational level

Parent coaching, family communication, mentoring, peer mediation where safe, restorative conversations, and repair with teachers.

## School level

Belonging, accessible curriculum, anti-bullying systems, fair discipline, staff training, flexible pathways, and student voice.

## Community level

Youth provision, sport, arts, libraries, safe transport, volunteering, employment experience, and culturally specific resources.

## Policy and institutional level

Housing, income support, disability access, interpreter services, exclusion reform, health access, and data monitoring for inequity.

The counsellor need not personally deliver every intervention. Professional competence includes referral, consultation, advocacy, letter writing, meeting facilitation, and follow-through.

# 9. Case Illustration: The “Resilient” Young Carer

**Presentation.** Ruth, sixteen, is praised by school for excellent attendance and care of her younger brother, who has complex medical needs. She asks for counselling because she has stopped applying to sixth-form courses and cries on the bus.

**Ecological formulation.** Ruth's strengths are real: organisation, empathy, academic ability, and a trusted aunt. The costs are also real. She administers medication before school, translates medical information, sleeps lightly in case her brother needs help, and has no unstructured peer time. Her mother works nights and is exhausted. A school attendance award has unintentionally become pressure never to be ill or late.

**Intervention.** The counsellor resists congratulating Ruth into further endurance. A care assessment is requested; respite and a young-carers service are explored; the hospital provides an adult interpreter and written care plan so Ruth is not the clinical intermediary. School offers a later start after hospital nights and a private route to report absence. Ruth joins a weekly art group protected from caring duties. Her mother and aunt create an emergency rota so Ruth is not the default responder.

Ruth applies to sixth form. The change is not that she becomes more resilient. The ecology becomes less dependent on extracting resilience from her.

## 10. School and Community Applications

Schools can build resilience by ensuring every student has at least one known trusted adult, predictable routines, fair discipline, accessible challenge, extracurricular belonging, anti-bullying enforcement, and early practical help. Universal provision matters because targeted programmes can stigmatise and miss students whose adversity is hidden.

Community resilience requires investment before crisis. Libraries, youth clubs, safe parks, transport, sports, arts, and culturally led organisations are mental health infrastructure even when not labelled as such. Counsellors should know local assets and help young people enter them, including addressing cost, travel, disability access, and fear of being an outsider.

## 11. Evidence and Limitations

Longitudinal research consistently identifies supportive relationships, effective caregiving, self-regulation, school engagement, and community resources as predictors of better adaptation under adversity. Interventions are most credible when they strengthen these ordinary systems.

Resilience research has limitations. Definitions and outcomes vary; many studies are observational; and positive adaptation in one domain may conceal distress in another. Cultural meanings of success differ: independence, academic achievement, family contribution, spiritual life, and collective wellbeing may be weighted differently, though this textbook does not prescribe faith-based goals.

The central ethical limitation is political. Resilience language can shift responsibility from institutions to children. The discipline is to pair every individual strength with an environmental question: What support makes this strength sustainable? What burden should adults remove?

## 12. Key Terms

**Key Terms at a Glance**

- **Resilience** — Successful adaptation under adversity through interacting individual and environmental processes.
- **Microsystem** — Immediate settings in which a young person directly participates.
- **Mesosystem** — Connections among microsystems, such as family–school communication.
- **Exosystem** — Indirect settings affecting development, such as parental employment conditions.
- **Macrosystem** — Cultural, legal, and economic structures shaping development.
- **Chronosystem** — The dimension of time, transitions, and historical context.
- **Promotive factor** — A resource associated with positive outcomes across risk levels.
- **Developmental cascade** — Spreading effects of change across domains and time.

## 13. Chapter Summary

Ecological and strengths-based practice widens the counsellor's field of vision. The young person remains an agent, but not an isolated one. Good practice identifies risk without fatalism, names strengths without exploitation, and intervenes at the level where change is both possible and just.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Why is resilience not a personality trait?
2. Apply all five ecological systems to school refusal.
3. Distinguish risk, protective, and promotive factors.
4. What hidden costs may accompany being labelled resilient?
5. Give three examples of ordinary magic in adolescent development.
6. How can a bus pass or timetable change be a counselling intervention?
7. When does strengths language become unethical?

## 15. Practice Activities

**Practice and Application**

### Activity 50.1 — Ecological map

Construct an ecological map for a fictional young carer, marking pressures, resources, power, and one intervention per level.

### Activity 50.2 — Strength specificity

Rewrite five vague strengths (“resilient”, “mature”, “coping well”) into observable capacities and the supports sustaining them.

### Activity 50.3 — Cascade planning

Identify one small change that could create a positive developmental cascade for a school-avoidant adolescent.

## 16. Annotated Reading Pathways

**For theory.** Bronfenbrenner provides structure; Masten and Ungar provide process and social ecology.

**For practice.** Saleebey's strengths perspective should be read alongside critical literature warning against responsibilising disadvantaged families.

**For schools.** Focus on belonging, trusted adults, fair discipline, practical barriers, and extracurricular access as measurable ecological conditions.

# Practice Laboratory and Extended Worked Example

This laboratory turns **Resilience, Ecological Systems, and Strengths-Based Practice** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is bronfenbrenner, Masten's resilience research, and the systematic mobilisation of protective factors. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is development, agency, identity, culture, relationships, motivation, and ecological resources. The mechanism to keep in view is how beliefs, social models, environments, power, belonging, and developmental timing interact to support or restrict change. The characteristic professional risk is individualising structural harm, stereotyping culture, or demanding resilience while leaving unsafe conditions unchanged. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in schools, families, peer contexts, community services, and culturally responsive practice, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Jonas is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Jonas's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Jonas explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on development, agency, identity, culture, relationships, motivation, and ecological resources. The first intervention is deliberately modest. Jonas chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Jonas has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Jonas can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Resilience, Ecological Systems, and Strengths-Based Practice

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Resilience, Ecological Systems, and Strengths-Based Practice** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Resilience, Ecological Systems, and Strengths-Based Practice** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Resilience, Ecological Systems, and Strengths-Based Practice**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Resilience, Ecological Systems, and Strengths-Based Practice to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

PART VIII

# Integration, Assessment, and Professional Practice

The working core of the course: how theories are integrated into a coherent stance, how cases are conceptualised, how progress is measured, and how the course itself is taught and assessed.

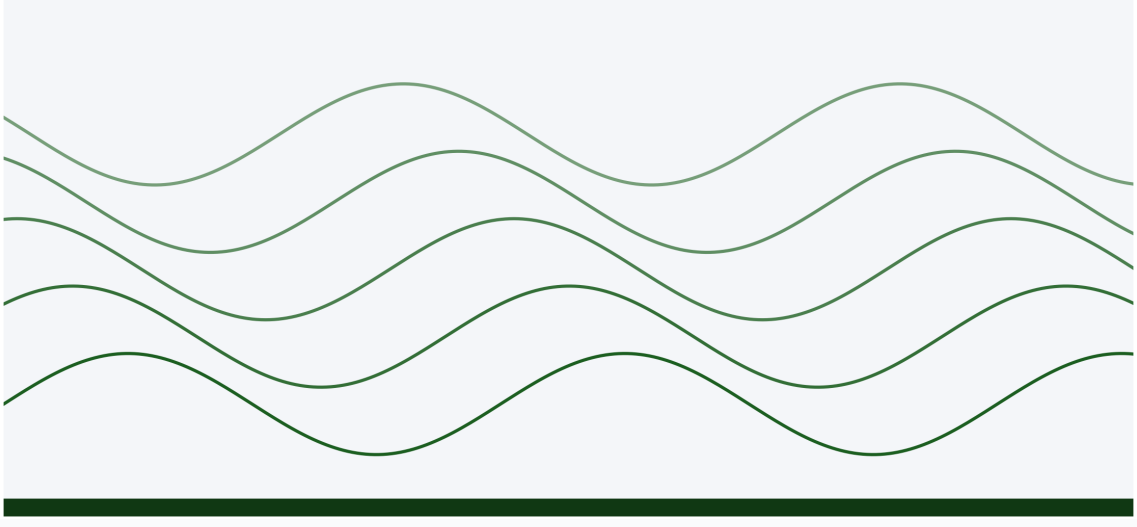
CHAPTERS IN THIS PART


51 Theoretical Integration and the Integrative Stance
52 Case Conceptualization and Treatment Planning
53 Assessment and Outcome Monitoring in Counseling
54 Termination, Evaluation, and the Ethics of Endings
55 Teletherapy and Technology in Counseling with Youth
56 School Counseling Practice and Consultation
57 Teaching CAC. 701: Session Plans and Facilitation
58 Assessment Blueprint, Assignments, and Rubrics

CHAPTER 51

# Theoretical Integration and the Integrative Stance

Technical eclecticism, assimilative integration, and the discipline of a coherent theory of change.



## Learning Objectives

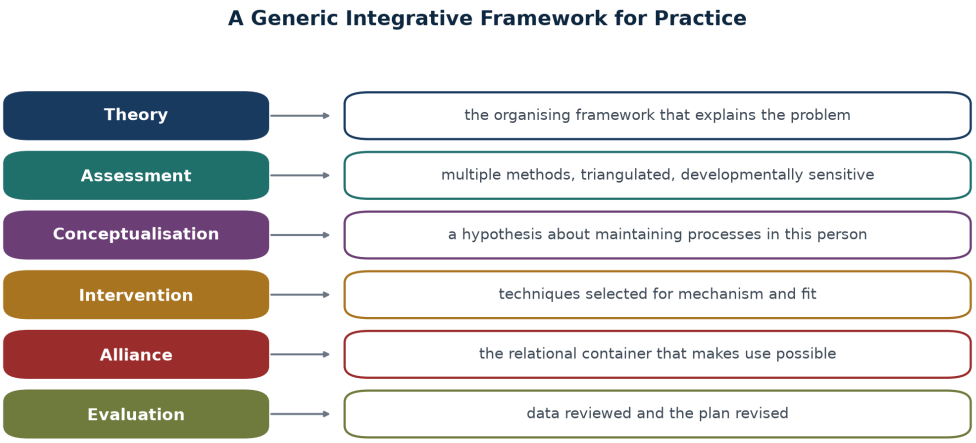
By the end of this chapter, the student should be able to:

- Distinguish theoretical integration, technical eclecticism, common-factors integration, and assimilative integration.
- Evaluate benefits and risks of combining theories.
- Formulate cases across affective, behavioural, cognitive, interpersonal, systemic, developmental, and cultural dimensions.
- Select interventions according to client need, evidence, mechanism, timing, and practitioner competence.
- Maintain coherence when moving between theories during treatment.
- Explain integration to clients and families without presenting a confusing mixture of jargon.
- Use supervision and outcome data to govern integrative choices.

## Chapter Overview

Few experienced counsellors practise one theory in pure form. A practitioner may build a person-centred relationship, use behavioural activation, teach grounding, involve parents, challenge a school practice, and help an adolescent re-author a shame-based story. The question is not whether integration occurs, but whether it is principled.

Unprincipled integration becomes a drawer of techniques: a worksheet because it is Tuesday, a chair exercise because training was recent, a genogram because the family seems complicated. Principled integration begins with formulation. What maintains this problem? What capacity needs building? What relationship or environment must change? What mechanism does each intervention engage? How will we know whether it helps?




*The framework is generic; the content of each row is supplied by the theory the counsellor has chosen to adopt.*

**Figure 51.43** An integrative framework organised around formulation, relationship, mechanisms of change, context, and review.

# 1. Forms of Integration

**Theoretical integration** attempts to combine concepts from two or more theories into a coherent framework. It is intellectually ambitious and difficult because theories disagree about persons, causation, and change.

**Technical eclecticism** selects techniques by evidence and fit without requiring theoretical unification. Its strength is pragmatism; its risk is incoherence.

**Common-factors integration** organises practice around alliance, empathy, expectations, goals, feedback, cultural responsiveness, and therapist effects. It explains why different therapies often produce comparable outcomes but does not by itself specify every technique.

**Assimilative integration** maintains a home theory while importing techniques from others in ways translated into the home framework. A psychodynamic clinician might use behavioural activation as a way to test relational expectations; a CBT clinician might use narrative externalisation to reduce shame before skills work.

# 2. Why Integrate?

Single theories illuminate different levels. Behavioural theory clarifies contingencies. Attachment theory clarifies relational expectations. Cognitive theory clarifies appraisal. Family systems clarify interaction patterns. Trauma theory clarifies state and safety. Cultural theory clarifies power and context.

Adolescent problems rarely sit on one level. School refusal may involve panic physiology, avoidance learning, perfectionistic beliefs, parental accommodation, bullying, sleep disturbance, learning difficulty, and a young person's identity as “the anxious one”. Integration allows sequencing rather than competition.

Integration also respects client preference. Some young people want practical tools; others need to tell a story; many need both at different times. Preference is not the only criterion, but it affects alliance and engagement.

# 3. Risks of Integration

Integration can dilute competence. A practitioner who knows fragments of ten models may deliver none well. It can also conceal contradiction: using unconditional acceptance while covertly engineering compliance; demanding emotional exposure before stabilisation; or asking parents to change while telling the adolescent every problem is systemic.

Other risks include unfalsifiability, technique shopping after setbacks, excessive complexity for the client, and cultural cherry-picking. Integration requires more discipline, not less.

# 4. A Principled Integrative Method

## Step 1: Describe before explaining

Record observable patterns, contexts, strengths, risks, and the client's own language.

## Step 2: Formulate across levels

Use the five-part structure described in Chapter 52: predisposing, precipitating, perpetuating, protective, and presentational factors across biological, psychological, relational, developmental, cultural, and institutional domains.

### Step 3: Identify mechanisms and targets

Translate formulation into mechanisms: avoidance, arousal, belief, skill deficit, reinforcement, family sequence, shame, isolation, unmet need.

### Step 4: Sequence

Safety and stabilisation precede exposure. Alliance and consent precede challenge. Parent capacity may precede adolescent skills. Practical barriers may precede insight.

### Step 5: Match intervention to mechanism

Select the least complex intervention likely to affect the mechanism, within competence and evidence.

### Step 6: Explain the plan

Use plain language: “First we will help sleep and mornings become steadier. Then we will test the fear step by step. Your parents will work on responses that accidentally keep the avoidance going.”

### Step 7: Measure and revise

If outcomes do not improve, revisit formulation rather than adding random techniques.

## 5. Integration across This Book

A trauma-informed stabilisation plan may draw on somatic grounding, attachment-based co-regulation, CBT sleep skills, school consultation, and family psychoeducation. An eating or self-harm presentation may require medical care, family work, emotion regulation, and risk planning. Integration is therefore also interprofessional.

Narrative externalisation can reduce shame before exposure. Motivational interviewing can prepare a young person for skills work. Social cognitive theory can explain why mastery experiences are designed. Mentalization can guide family communication during behavioural changes. The theories become complementary when each is assigned a clear job.

## 6. Further Reading for This Chapter


- Norcross, J. C., & Goldfried, M. R. (Eds.). (2019). *Handbook of Psychotherapy Integration* (3rd ed.).
- Wachtel, P. L. (1997). *Psychoanalysis, Behavior Therapy, and the Relational World*.
- Lazarus, A. A. (1997). *Brief but Comprehensive Psychotherapy: The Multimodal Way*.
- Prochaska, J. O., & Norcross, J. C. (2018). *Systems of Psychotherapy: A Transtheoretical Analysis* (9th ed.).
- Castonguay, L. G., & Beutler, L. E. (Eds.). (2006). *Principles of Therapeutic Change that Work*.

## 7. Case Formulation as the Integrative Home

Integration needs a home more stable than fashion. Case formulation provides it. A good formulation is collaborative, developmental, cultural, testable, and linked to intervention.

Consider a sixteen-year-old with low mood, school absence, cannabis use, and conflict at home. A purely cognitive plan may miss sleep, peer belonging, parental exhaustion, and the function of cannabis. A purely systemic plan may miss depressive cognition and suicide risk. An integrative formulation can hold all of these while sequencing work:

1. risk and medical assessment;
2. sleep and morning routine;
3. motivational interviewing about cannabis and attendance;
4. behavioural activation and graded school return;
5. parent sessions on escalation and accommodation;
6. cognitive work on perfectionism and shame;
7. school adjustments and careers support.

The plan is integrated because each element has a reason and an order, not because many brand names appear.

## 8. Switching, Blending, and Sequencing

**Switching** means changing approach when phase or problem changes—for example, from stabilisation to trauma-focused work.

**Blending** means using compatible elements together—for example, narrative language within behavioural activation.

**Sequencing** means ordering interventions by dependency: safety before disclosure, parent session before joint communication, sleep before demanding exposure.

Confusion arises when practitioner switches because they are anxious, bored, or avoiding a rupture. Supervision should ask: What formulation changed? What mechanism is now targeted? What did the client consent to? What outcome will test the change?

## 9. Competence and Scope

Practitioners should distinguish between using an idea, using a technique, and delivering a treatment model. Reading about EMDR does not make one an EMDR practitioner. Borrowing scaling questions does not make a session solution-focused therapy. A grounding exercise does not constitute trauma treatment.

Training, supervised practice, fidelity, and referral pathways matter. Ethical integration includes saying: “I can help with stabilisation and family communication; for trauma processing I will refer or work jointly with a trained clinician.”

Documentation should reflect actual practice. Do not record model names for prestige. Record formulation, intervention, rationale, consent, response, and review.

## 10. Evidence and Evaluation

Comparative outcome research often finds modest differences among bona fide therapies, though specific treatments have advantages for specific disorders and outcomes. Integration should therefore be guided by disorder-specific evidence, mechanisms, client preference, developmental fit, and monitoring—not by the assumption that more techniques equal more benefit.

Routine outcome monitoring is especially important in integrative work because drift is easy. If measures and goals do not improve, options include strengthening alliance, revising formulation, increasing fidelity, changing sequence, involving systems, consulting, or referring. Adding another technique is only one possibility.

## 11. Cultural and Ethical Considerations

Integration can become cultural appropriation when practices are detached from context and used as exotic techniques. The practitioner should understand origin, purpose, evidence, and client preference, and should avoid importing religious or spiritual practices into a secular counselling frame unless the client independently identifies them as personal resources and professional boundaries permit appropriate acknowledgement without counsellor-led religious activity.

Client consent should cover major shifts in method. A young person who agreed to practical coaching may experience unexpected trauma exploration as betrayal. Parents should understand family involvement, and schools should receive only agreed information.

## 12. Key Terms

**Key Terms at a Glance**

- **Assimilative integration** — Importing ideas or techniques into a coherent home theory.
- **Common factors** — Relationship, expectancy, goal, feedback, and context elements shared across therapies.
- **Technical eclecticism** — Selecting techniques by fit and evidence without unifying theories.
- **Theoretical integration** — Combining concepts from multiple theories into a coherent framework.
- **Mechanism of change** — The process by which an intervention produces improvement.
- **Treatment drift** — Gradual departure from effective practice without formulation-based reason.

## 13. Chapter Summary

The integrative stance is disciplined pluralism. It respects the partial truth of theories, assigns each intervention a mechanism and a place in sequence, tells the client what is happening and why, and uses outcomes to correct course. Integration is not doing everything; it is knowing why one is doing this, now, with this young person.

## 14. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish four forms of integration.
2. What questions prevent technical eclecticism from becoming incoherent?
3. Build a sequenced integrative plan for school refusal.
4. When is switching approach clinically justified rather than drift?
5. Why does integration require greater competence, not broader superficiality?
6. How should a major change in method be consented?
7. What outcome evidence should trigger reformulation?

## 15. Practice Activities

**Practice and Application**

### Activity 51.1 — Mechanism match

List five interventions from earlier chapters and identify the mechanism each targets.

### Activity 51.2 — Sequence defence

Write a ten-session plan for adolescent depression with self-harm risk. Defend the order of interventions.

### Activity 51.3 — Integration audit

Review a fictional session transcript and mark where theory changes are coherent, confusing, or ethically under-consented.

## 16. Annotated Reading Pathways

**For integration theory.** Norcross and Goldfried provide the major map of integration movements.

**For practice.** Multimodal and transtheoretical approaches show how assessment domains and readiness shape intervention choice.

**For evidence.** Read principles-of-change literature alongside disorder-specific guidelines so that common factors do not erase real differences among treatments.

# Practice Laboratory and Extended Worked Example

This laboratory turns **Theoretical Integration and the Integrative Stance** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is technical eclecticism, assimilative integration, and the discipline of a coherent theory of change. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and**

**eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Priya is fifteen and is seen in a school counselling service for low mood and withdrawal from friends. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Priya's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Priya explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Priya chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Priya has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Priya can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Theoretical Integration and the Integrative Stance

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Theoretical Integration and the Integrative Stance** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Theoretical Integration and the Integrative Stance** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

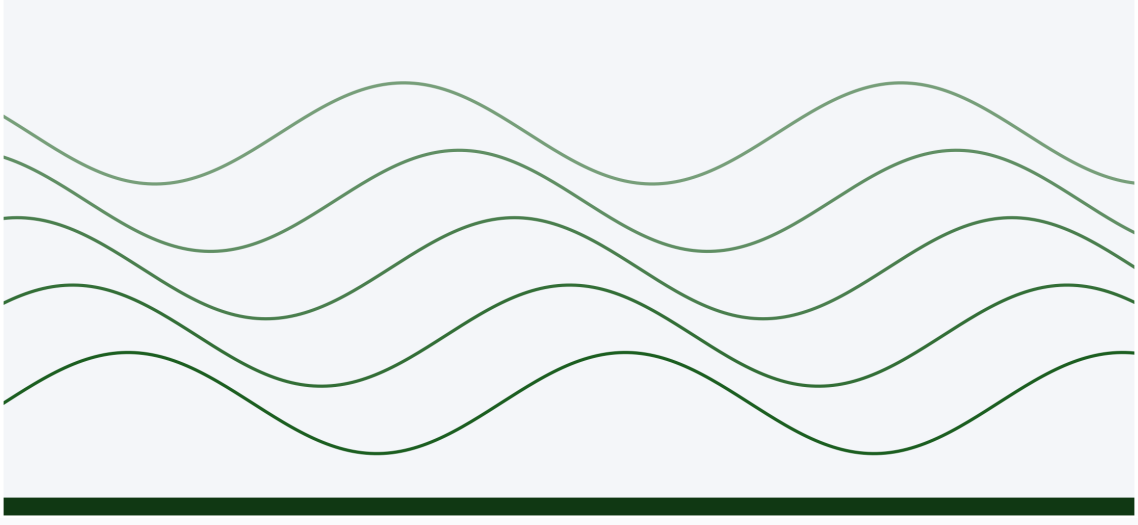
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Theoretical Integration and the Integrative Stance**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Theoretical Integration and the Integrative Stance to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 52

# Case Conceptualization and Treatment Planning

The five-part formulation, the mechanism of change, and the written treatment plan.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Distinguish description, diagnosis, formulation, and treatment plan.
- Construct a developmental, cultural, relational, and evidence-informed case conceptualization.
- Use predisposing, precipitating, perpetuating, protective, and presenting factors.
- Translate formulation into goals, interventions, sequencing, responsibilities, and review criteria.
- Write SMART but humane goals with adolescents and families.
- Identify risk, safeguarding, medical, educational, and social needs within a plan.
- Revise formulation in response to outcome data and new information.

## Chapter Overview

A diagnosis can name a pattern; it cannot by itself explain why this young person, in this family, in this school, at this time, is struggling in this way. Case conceptualization—also called formulation—is the bridge between assessment and treatment. It organises information into a testable explanation and identifies where change is possible.

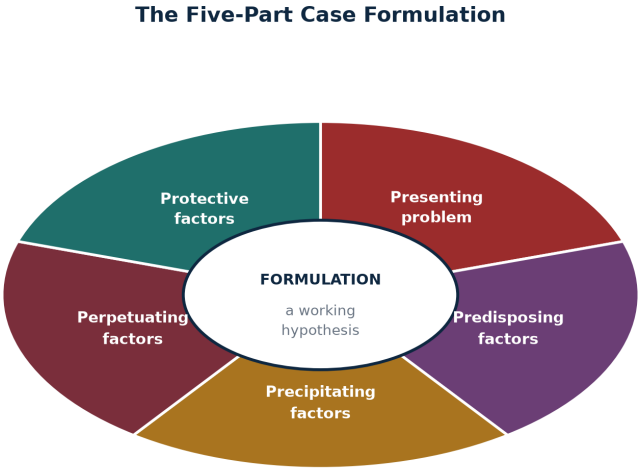



**Figure 52.44** The five-part formulation: presenting issues, predisposing factors, precipitating events, perpetuating mechanisms, and protective factors.

A good formulation is neither a biography nor a list of deficits. It is a concise, collaborative hypothesis that can be shared in developmentally appropriate language: what happened, what it meant, what keeps the problem going, what strengths exist, and what we will try first.

## 1. Description, Diagnosis, and Formulation

**Description** records what is happening: symptoms, behaviour, context, frequency, duration, severity, and impact.

**Diagnosis** classifies patterns according to recognised criteria. It can guide evidence, communication, eligibility, and prognosis, but may also stigmatise or obscure context.

**Formulation** explains development and maintenance for an individual in context. It includes strengths and systems as well as symptoms.

**Treatment plan** specifies goals, interventions, people, timeframe, measures, and review.

Confusing these levels leads to poor practice. “She avoids school because she has anxiety” is circular if anxiety is inferred only from avoidance. A formulation might instead identify panic sensations, catastrophic appraisal, avoidance relief, parental accommodation, bullying, sleep disturbance, and academic gaps—each suggesting intervention.

## 2. Sources of Information

Formulation draws on interviews with the young person, parents or carers, school information, previous records, observations, standardised measures, medical information, and the young person's creative or written expression. Disagreement between informants is not noise to erase; it reveals setting-specific patterns and different perspectives.

The counsellor should distinguish observation, report, inference, and speculation in records. “Mother reports that he refuses” is a report. “He appeared tense when discussing mathematics” is observation. “He may fear humiliation” is inference. “He is manipulative” is an unhelpful character judgement unless operationalised and evidenced.

## 3. The Five-Part Framework

### Presenting factors

Current concerns, symptoms, risks, functional impact, and the reason help is sought now.

### Predisposing factors

Temperament, neurodevelopment, early relationships, prior adversity, learning history, health, family patterns, cultural context, and earlier coping.

### Precipitating factors

Recent losses, transitions, conflicts, illness, bullying, academic pressure, family changes, or developmental demands that exceeded resources.

### Perpetuating factors

Avoidance, reinforcement, accommodation, insomnia, substance use, rumination, conflict cycles, inaccessible teaching, untreated pain, discrimination, and service fragmentation.

### Protective factors

Relationships, skills, interests, cultural and community resources, school belonging, prior successes, family commitment, practical resources, and hope.

The model becomes powerful when arrows are drawn between parts. A precipitant matters because of predisposition; a perpetuating loop explains why distress persists after the precipitant ends.

## 4. Levels of Formulation

A comprehensive adolescent formulation considers:

- biological and physical health;
- cognition, emotion, behaviour, and identity;
- attachment and family interaction;
- peers, school, and digital contexts;
- culture, language, discrimination, and community;
- material conditions and service systems;
- development and time.

Not every level requires intervention. The art lies in choosing leverage points: factors that are influential, changeable, acceptable to the client, and within available competence.

## 5. Collaborative Formulation

Young people should recognise themselves in the formulation without feeling reduced by it. Use their words where possible. Draw diagrams together. Offer uncertainty: “This is our best current map, not a final truth. What fits? What is missing? What feels wrong?”

Parents may need a parallel version focused on interactions and support, without disclosing confidential adolescent material. Schools need a functional, privacy-respecting summary focused on responses and adjustments.

A formulation that cannot be communicated is unfinished.

## 6. Further Reading for This Chapter


- Johnstone, L., & Dallos, R. (Eds.). (2014). *Formulation in Psychology and Psychotherapy* (2nd ed.).
- Carr, A. (2016). *The Handbook of Child and Adolescent Clinical Psychology* (3rd ed.).
- Kuyken, W., Padesky, C. A., & Dudley, R. (2009). *Collaborative Case Conceptualization*.
- Persons, J. B. (2008). *The Case Formulation Approach to Cognitive-Behavior Therapy*.
- Eells, T. D. (Ed.). (2022). *Handbook of Psychotherapy Case Formulation* (3rd ed.).


## 7. From Formulation to Goals

Goals should be meaningful, observable enough to review, achievable in sequence, and owned by the young person and family where appropriate. SMART criteria are useful if not applied mechanically. “Reduce

anxiety" is vague. "Attend first two lessons on four of five days for two weeks, using agreed entry route and break card" is reviewable. But human goals may also include feeling safer with mother, having one friend to eat with, or being able to sleep without checking locks. These can be scaled and described.

Distinguish:

- **Safety goals:** non-negotiable protection and risk reduction.
- **Stabilisation goals:** sleep, routine, regulation, attendance, basic functioning.
- **Change goals:** skills, relationships, beliefs, family patterns, academic participation.
- **Developmental goals:** autonomy, friendships, identity exploration, qualifications, transitions.
- **System goals:** school adjustments, coordinated care, parent support, practical resources.

A plan overloaded with goals becomes another experience of failure. Three to five active priorities are usually enough.

## 8. Treatment Planning

A complete plan states:

1. formulation summary;
2. goals and rationale;
3. interventions and who delivers them;
4. client, family, school, and professional responsibilities;
5. frequency, setting, and expected duration;
6. consent, assent, confidentiality, and information sharing;
7. risk and crisis arrangements;
8. measures and review dates;
9. criteria for intensifying, changing, referring, or ending;
10. practical barriers and solutions.

Sequencing is a clinical decision. Safety and engagement come before demanding processing. Parent work may precede joint sessions. Sleep and medical assessment may precede exposure. Skills may precede trauma narration. School changes may need to occur while individual therapy is still early.

## 9. Worked Example

**Presentation.** Amara, fifteen, has panic symptoms, school absence, falling grades, and nightly arguments.

**Formulation.** Predisposing: high sensitivity, perfectionism, previous bullying. Precipitating: school transition and public panic attack in assembly. Perpetuating: avoidance, reassurance seeking, mother leaving work to collect her, sleep reversal, academic backlog, teachers questioning absence in front of peers. Protective: strong writing ability, one trusted friend, committed grandmother, mother willing to change responses, school counsellor available.

**Plan.** Medical review; shared formulation; sleep routine; parent session on reassurance and collection thresholds; graded school return beginning with counsellor office and two lessons; interoceptive and situational exposure with trained practitioner; private teacher check-ins; academic triage so backlog does not become a second threat; weekly attendance and panic ratings; review after four weeks.

Notice that the plan is neither "CBT" as a slogan nor a collection of unrelated supports. Each item maps to a perpetuating factor.

## 10. Risk and Safeguarding in Plans

Risk formulation should identify static and dynamic factors, triggers, warning signs, protective factors, likely scenarios, and actions. Risk is not a score alone. Plans must name who does what, when, and how the young person can seek help.

Safeguarding concerns require action under local law and policy. Treatment planning cannot delay protection. Where parents are part of risk, information sharing and session structure change accordingly.

Plans should also consider risk created by treatment itself: exposure distress, family conflict after sessions, confidentiality breaches, transport home while dysregulated, online contact boundaries, and deterioration between appointments.

## 11. Review and Reformulation

Review asks:

- What improved, worsened, or stayed the same?
- Which formulation predictions were supported or disconfirmed?
- Was the intervention delivered as planned?
- Did the environment change?
- What does the young person say helped or harmed?
- Should intensity, focus, participants, or provider change?

Lack of progress is not automatically resistance. It may indicate wrong target, insufficient dose, poor alliance, unaddressed practical barriers, medical issues, family overload, or a formulation that flatters the professional's preferred theory.

Endings should be considered from the beginning. A plan that creates indefinite dependency without developmental purpose fails even if sessions feel warm.

## 12. Documentation Quality

Records should be accurate, respectful, and written as if the young person may read them—because in many systems they can. Avoid labels without evidence, speculative motives stated as fact, and copying forward outdated risk information. Date formulations, note whose views they represent, and record disagreements.

Treatment plans shared across agencies should include the minimum necessary information. Consent and legal basis for sharing must be clear.

## 13. Cultural and Ethical Considerations

Formulation can reproduce bias when culture is treated as pathology, poverty as parenting failure, or institutional harm as individual disorder. Include discrimination and material context explicitly, and ask the young person and family how they explain the problem.

Power matters in goal setting. A school may want compliance, a parent obedience, a service discharge, and the young person safety or freedom. The plan should identify whose goal each target serves and negotiate conflicts transparently.

Consent is ongoing. As formulation changes, the rationale for intervention should be re-explained. Young people should know what records exist, who can see summaries, and how to correct factual errors.

## 14. Key Terms

**Key Terms at a Glance**

- **Formulation** — A collaborative, testable explanation of how difficulties developed and are maintained.
- **Diagnosis** — Classification of a pattern according to recognised criteria.
- **Perpetuating factors** — Processes that maintain current difficulties.
- **Protective factors** — Relationships, capacities, and resources that buffer adversity.
- **Leverage point** — A changeable factor likely to influence the wider system.
- **Risk formulation** — Contextual explanation of risk scenarios, triggers, safeguards, and responses.

## 15. Chapter Summary

Case conceptualization turns information into judgement and judgement into a plan. Its quality is measured not by theoretical elegance but by whether it helps a real young person, family, and service decide what to do next, in what order, with what consent, and how they will know whether it worked.

## 16. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish description, diagnosis, formulation, and plan.
2. Why is “avoidance because of anxiety” potentially circular?
3. Construct a five-part formulation for examination panic.
4. Convert three vague goals into reviewable goals without making them mechanistic.
5. What should happen when outcome data disconfirm a formulation?
6. How can school and parent goals conflict with the adolescent's goals?
7. Write a record entry that separates observation, report, and inference.

## 17. Practice Activities

**Practice and Application**

### Activity 52.1 — Five-part map

Create a five-part formulation for a fictional adolescent with low mood and family conflict. Draw arrows between factors.

### Activity 52.2 — Plan translation

Turn each perpetuating factor into one intervention, one responsible person, and one review measure.

### Activity 52.3 — Young-person version

Rewrite a professional formulation in 120 words of respectful language suitable for a fourteen-year-old.

## 18. Annotated Reading Pathways

**For formulation.** Johnstone and Dallos offer cross-theoretical depth; Kuyken and colleagues emphasise collaboration.

**For treatment planning.** Pair formulation texts with local safeguarding, consent, record-keeping, and service documentation standards.

**For practice.** Judge every formulation by whether it changes a decision. If it does not, it may be biography rather than formulation.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Case Conceptualization and Treatment Planning** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the five-part formulation, the mechanism of change, and the written treatment plan. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation,

planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

## A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach,

peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tunde is fifteen and is seen in a community adolescent clinic for grief following a major family change. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tunde's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tunde explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Tunde chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tunde has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tunde can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the

young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language;

the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Case Conceptualization and Treatment Planning

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Case Conceptualization and Treatment Planning** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Case Conceptualization and Treatment Planning** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

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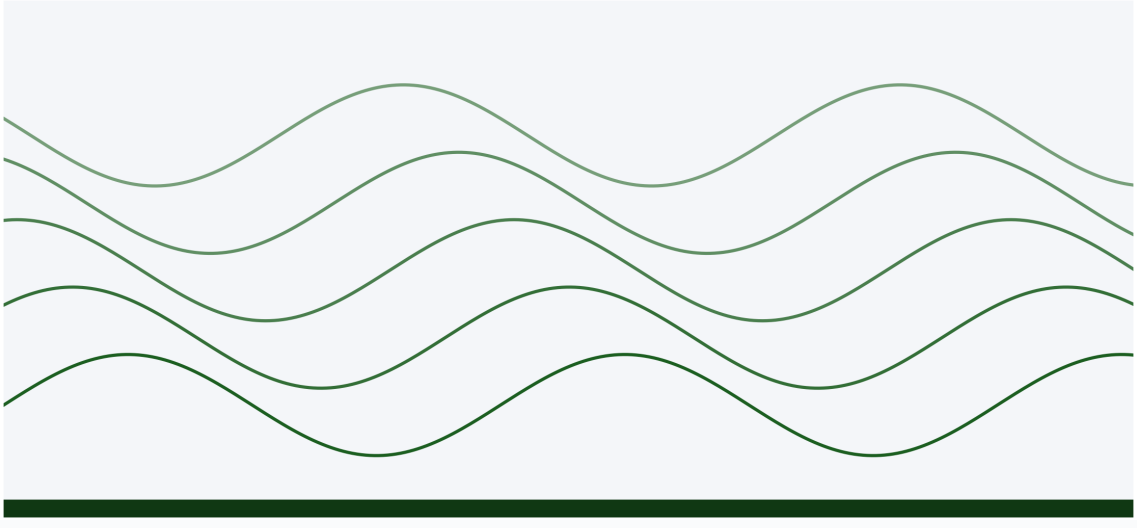
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Case Conceptualization and Treatment Planning**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Case Conceptualization and Treatment Planning to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 53

# Assessment and Outcome Monitoring in Counseling

Instruments, session notes, measurement-based care, and the ethical use of assessment data.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain purposes and limits of assessment in adolescent counselling.
- Select interviews, observations, rating scales, diaries, and functional measures appropriate to a referral question.
- Interpret scores using reliability, validity, norms, measurement error, and cultural fairness.
- Use routine outcome monitoring and session feedback to improve care.
- Distinguish symptom, functioning, wellbeing, personal recovery, and client-defined outcomes.
- Communicate assessment findings without reducing a young person to a score.
- Apply ethical standards for consent, privacy, data storage, testing, and feedback.

## Chapter Overview

Assessment is not the collection of everything knowable. It is the disciplined attempt to answer specific questions well enough to make decisions. What is happening? How severe and persistent is it? In which settings? What maintains it? What strengths and risks exist? What intervention is indicated? Is change occurring? Could assessment itself cause harm?

Outcome monitoring extends assessment across treatment. Brief, repeated measures can reveal improvement, stasis, deterioration, alliance problems, and mismatch between professional impressions and client experience. Used collaboratively, measures become conversation tools. Used mechanically, they become surveillance.

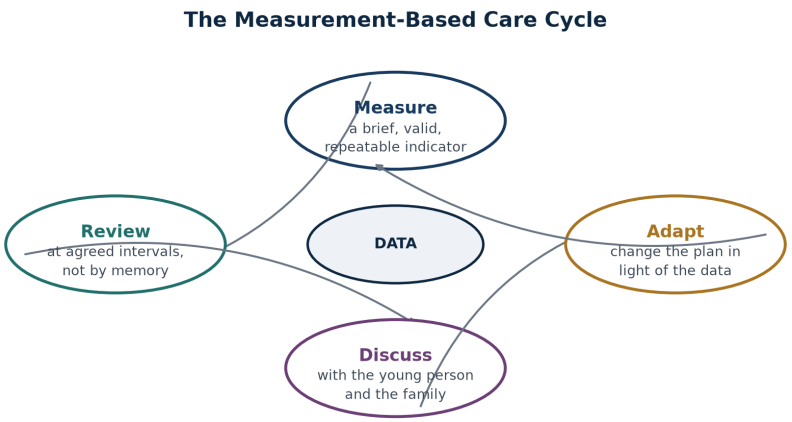



**Figure 53.45** Measurement-based care: assess, share feedback, adjust intervention, and reassess in a continuous loop.

## 1. Purposes of Assessment

Assessment may serve screening, diagnosis, formulation, risk identification, treatment selection, progress monitoring, service evaluation, safeguarding, education planning, or research. Each purpose requires

different evidence.

A screening tool identifies who needs fuller assessment; it does not diagnose. A symptom scale measures severity in a domain; it does not explain cause. An intelligence or attainment test answers specific educational questions; it does not summarise worth or potential. A risk scale may support structured judgement; it cannot predict an individual's future with certainty.

Students should always be able to complete the sentence: “This assessment is being used to decide…” If no decision follows, reconsider whether it is necessary.

## 2. Multi-Method, Multi-Informant Assessment

Adolescents behave differently across contexts. Self-report captures internal experience. Parent report captures home functioning and developmental history. Teacher report captures classroom participation and peer context. Observation captures interaction. Records capture attendance, attainment, incidents, and prior interventions.

Discrepancies are informative. A parent may report severe anxiety that teachers do not see because the student masks at school and collapses at home. A teacher may report inattention not reported by parents because classroom demands differ. Low agreement does not mean one informant is lying.

Interviews should be developmentally adapted: concrete examples, timelines, scaling, drawings, card sorts, digital timelines where appropriate, and permission to pause. Avoid leading questions in safeguarding and forensic contexts and follow local protocols.

## 3. Psychometric Foundations

**Reliability** concerns consistency: internal consistency, test–retest stability, and inter-rater agreement.

**Validity** concerns whether evidence supports a particular interpretation and use. A scale valid for screening adolescents in one population is not automatically valid for diagnosis in another language, culture, age group, or disability population.

**Norms** describe comparison samples. They must be relevant. A percentile is not a percentage correct and not a measure of personal value.

**Measurement error** means small score changes may not be meaningful. Reliable change indices and confidence intervals help, but clinical judgement and functional evidence remain essential.

**Sensitivity and specificity** matter in screening. A highly sensitive tool may identify many false positives; a specific tool may miss cases. Base rates affect predictive value.

**Cultural and linguistic fairness** requires more than translation. Concepts, idioms, response styles, family structures, and experiences of discrimination can alter meaning. Use validated adaptations and qualified interpreters; do not use children as interpreters for sensitive parental material.

## 4. Domains of Assessment

A comprehensive adolescent assessment may include:

- presenting concerns and client goals;

- mood, anxiety, trauma, psychosis, eating, substance use, sleep, and physical health;
- neurodevelopment, cognition, language, learning, and sensory needs;
- identity, culture, discrimination, and belonging;
- family relationships, caregiving capacity, and home safety;
- peer relationships, bullying, digital life, and exploitation risk;
- school functioning, attendance, attainment, and support;
- strengths, interests, coping, and previous helpful responses;
- risk to self, risk to others, vulnerability, and protective factors.

Comprehensive does not mean every domain in every case. It means the counsellor knows what has been omitted and why.

## 5. Functional and Idiographic Measures

Standardised scales compare a young person with groups. Idiographic measures track what matters to this person: number of panic episodes, nights slept in own bed, arguments lasting more than ten minutes, lessons attended, messages sent before checking, meals eaten with family, or a 0–10 rating of "feeling able to cope at school".

Goal-based outcomes invite the young person to define goals and rate progress. Session rating scales sample alliance and fit. Symptom diaries reveal timing and triggers.

Combining nomothetic and idiographic measures prevents two errors: treating population scores as the whole person, and treating personal impressions as immune to bias.

## 6. Further Reading for This Chapter


- American Educational Research Association, American Psychological Association, & National Council on Measurement in Education. (2014). *Standards for Educational and Psychological Testing*.
- Hunsley, J., & Mash, E. J. (Eds.). (2018). *A Guide to Assessments that Work* (2nd ed.).
- Lambert, M. J. (2010). *Prevention of Treatment Failure*.
- Wolpert, M., Görzig, A., Deighton, J., et al. (2015). Comparison of indices of clinically meaningful change in child mental health. *Child and Adolescent Mental Health*.
- Weisz, J. R., & Gray, H. A. (2008). Evidence-based psychotherapy in youth mental health. *Journal of Child Psychology and Psychiatry*.


## 7. Routine Outcome Monitoring

Routine outcome monitoring usually involves brief measures at intake, periodically during treatment, and at ending. Feedback is discussed with the young person and, where appropriate, family. Graphs can make change visible, including temporary worsening during exposure or family restructuring.

Evidence in youth services suggests monitoring can improve engagement and outcomes, especially when practitioners actually review feedback and adjust care. Collecting data without discussion wastes time and

may damage trust.

Choose measures that are:

- relevant to goals and population;
- brief enough to complete honestly;
- validated for age, language, and context where possible;
- sensitive to meaningful change;
- accessible in format and reading level;
- interpreted alongside functioning and client narrative.

Do not continue an ineffective treatment indefinitely because the relationship is pleasant. Do not terminate solely because a score crosses a threshold while functional evidence and risk point elsewhere.

## 8. Feedback Conversations

Feedback should be collaborative, plain-language, and developmentally appropriate. Start with the young person's view. Share what the measure can and cannot say. Invite disagreement.

A useful structure is:

1. “What has felt better, worse, or unchanged?”
2. “This scale suggests sleep and worry improved, but school functioning did not. Does that fit?”
3. “What might explain the difference?”
4. “What should we keep, change, or ask school or family to do?”
5. “When shall we check again?”

Parents may receive summary feedback consistent with consent and confidentiality. Schools should receive functional recommendations rather than raw clinical scores unless there is a clear, agreed purpose.

## 9. Risk Assessment

Risk assessment combines history, current presentation, context, and protective factors. Structured professional judgement is preferable to either unstructured intuition or blind score use.

Record risk formulation, not merely “low/medium/high”. Identify scenarios: What might happen? Under what triggers? How soon? How severe? What has prevented harm so far? What will each person do?

Risk changes. Reassess after disclosures, losses, conflict, substance use, service transitions, anniversaries, online events, and treatment ruptures. Safety plans should be accessible to the young person, written in their language, and reviewed rather than filed as proof that risk was “managed”.

## 10. Assessment in Schools and Services

School counsellors may contribute to special educational needs assessment, behaviour support, attendance planning, safeguarding, and wellbeing monitoring. Their role must remain distinct from discipline. Students should know whether information may enter school systems.

Service-level dashboards should monitor access, waiting times, completion, outcomes, deterioration, and equity by relevant demographic and access variables. A service can show average improvement while underserving disabled students, minority groups, or those with unstable housing. Aggregate success can conceal unequal benefit.

Assessment burden matters. Repeated long questionnaires can exhaust families and bias data toward those with time, literacy, digital access, and trust in institutions. Short forms, assistance, translated materials, and alternatives are equity measures.

## 11. Case Illustration: When the Score Improved but Life Did Not

**Presentation.** Jonah, sixteen, completes a depression scale every four weeks. His score improves from severe to moderate. The service proposes discharge.

**Review.** Jonah reports sleeping slightly better after medication review, but he has stopped attending college, no longer sees friends, and spends most days gaming in a dark room. His mother says arguments are fewer because “we've stopped asking anything of him.” The symptom score improved, but participation and developmental functioning worsened.

**Reformulation.** The original plan treated depressive symptoms without sufficiently targeting avoidance, academic backlog, peer withdrawal, and family accommodation. The revised plan adds behavioural activation, graded college return, a peer activity, family sessions on expectations, and goal-based measures for attendance and social contact.

The case illustrates why outcome is multidimensional. A score is evidence, not a verdict.

## 12. Ethics and Data Protection

Assessment requires informed consent or assent appropriate to age and capacity, explanation of purpose, voluntary participation where applicable, and clear limits. Some safeguarding or court-ordered assessments limit choice; honesty about compulsion is essential.

Test security, practitioner competence, storage, access, retention, and secondary use must follow professional and legal standards. Digital platforms introduce risks involving servers, metadata, shared devices, and automated scoring. Artificial intelligence tools should not be used to diagnose or make high-stakes decisions without validated evidence, human accountability, transparency, and local approval.

Reports should be respectful and readable. Avoid copying deficit language forward across years. Correct factual errors. Share findings in time to influence decisions.

## 13. Key Terms

**Key Terms at a Glance**

- **Reliability** — Consistency of measurement.
- **Validity** — Degree to which evidence supports an interpretation and use of a score.
- **Screening** — Brief identification of people who may need fuller assessment.
- **Idiographic measure** — A measure tailored to an individual's symptoms, goals, or behaviours.
- **Nomothetic measure** — A standardised measure permitting group comparison.
- **Reliable change** — Change larger than expected from measurement error.
- **Routine outcome monitoring** — Repeated measurement and feedback during care.
- **Structured professional judgement** — Risk assessment combining evidence-based factors with contextual formulation.

## 14. Chapter Summary

Good assessment answers decision-relevant questions using several methods and perspectives, acknowledges uncertainty, and respects the person beyond the score. Outcome monitoring makes counselling accountable to lived change: symptoms, functioning, relationships, participation, safety, and goals chosen by the young person.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Why does a screening score not establish diagnosis?
2. Explain reliability and validity with a school wellbeing scale example.
3. Why can parent, teacher, and adolescent reports legitimately disagree?
4. Design one standardised and one idiographic outcome measure for panic-related school avoidance.
5. What should happen when symptom scores improve but functioning worsens?
6. Identify three equity problems in digital outcome collection.
7. Write a feedback statement that shares uncertainty respectfully.

## 16. Practice Activities

**Practice and Application**

### Activity 53.1 — Measure selection

For adolescent social anxiety, select a screening tool, symptom measure, functional measure, and client-defined goal measure. Justify each.

### Activity 53.2 — Feedback role-play

Practise sharing a graph showing deterioration during exposure therapy without alarming or falsely reassuring a parent.

### Activity 53.3 — Report revision

Rewrite a deficit-heavy assessment paragraph into accurate, respectful, formulation-linked language.

## 17. Annotated Reading Pathways

**For psychometrics.** The testing standards are essential before using standardised instruments beyond screening.

**For youth outcomes.** Read child mental health outcome literature on reliable change, goal-based outcomes, and the limits of symptom-only endpoints.

**For implementation.** Monitoring works when feedback changes decisions. Study service examples where data review is built into supervision and team meetings.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Assessment and Outcome Monitoring in Counseling** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is instruments, session notes, measurement-based care, and the ethical use of assessment data. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Sofia is fifteen and is seen in a family support service for sleep disruption, irritability, and falling participation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Sofia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Sofia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through

repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Sofia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Sofia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Sofia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Assessment and Outcome Monitoring in Counseling

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Assessment and Outcome Monitoring in Counseling** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Assessment and Outcome Monitoring in Counseling** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

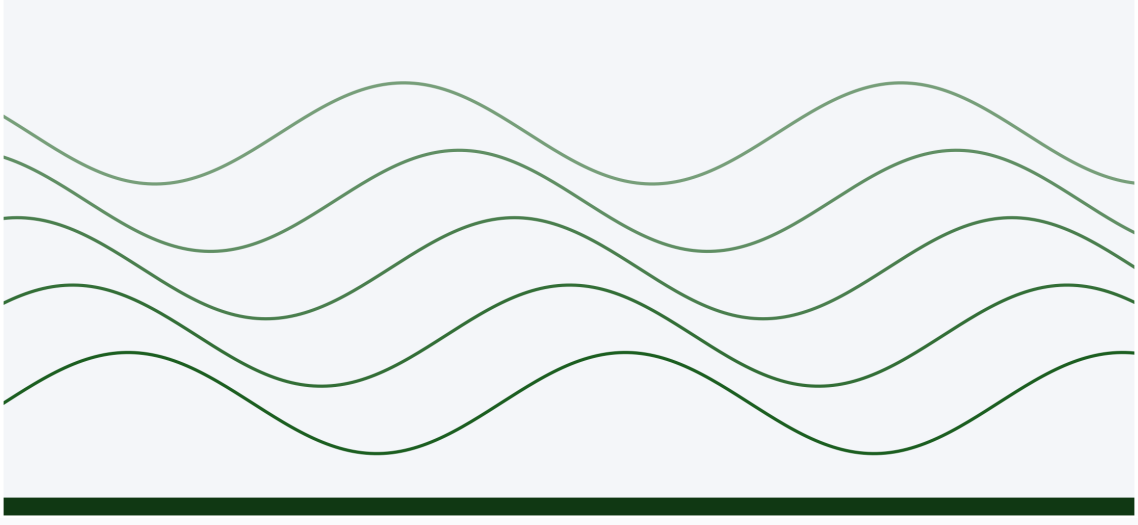
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Assessment and Outcome Monitoring in Counseling**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Assessment and Outcome Monitoring in Counseling to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 54

# Termination, Evaluation, and the Ethics of Endings

Planned termination, relapse prevention, transfer, and the evaluation of outcome.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Explain why endings are active therapeutic events rather than administrative afterthoughts.
- Distinguish planned termination, transfer, interruption, dropout, and service-imposed ending.
- Prepare adolescents and families for endings from the beginning of treatment.
- Evaluate progress, consolidate learning, and plan relapse prevention.
- Manage practitioner transitions and endings caused by placement, funding, school leaving, or risk escalation.
- Address abandonment, dependency, gratitude, anger, and unfinished business ethically.
- Write closing summaries and referrals that protect continuity and dignity.

## Chapter Overview

Every counselling relationship ends. It may end because goals are achieved, time expires, school terms turn, a practitioner leaves, a family moves, risk requires another service, or the young person stops attending. Endings can consolidate autonomy or repeat experiences of rejection and disappearance. Ethical practice therefore treats termination as part of treatment from the first session.

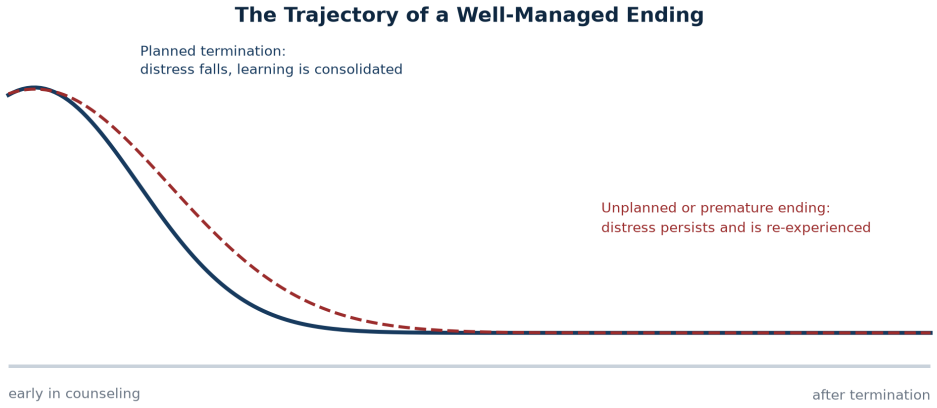



**Figure 54.46** The termination path: review beginnings, consolidate change, rehearse future challenges, plan support, and close with a clear door back to help.

## 1. Why Endings Matter Developmentally

Adolescents are already negotiating separations: changing schools, shifting friendships, leaving home, losing relatives, moving between care placements, and approaching adult services. Counselling endings can echo these transitions. A young person with abandonment sensitivity may test the ending by missing sessions, devaluing progress, becoming suddenly worse, or leaving first. These responses are not manipulation by default; they are information about attachment, control, and grief.

A planned ending offers a different experience: the date is known, feelings can be named, progress is reviewed, anger and sadness are permitted, and help-seeking in the future is framed as competence rather

than failure.

## 2. Types of Ending

**Planned mutual termination** follows goal review and relapse planning.

**Time-limited ending** is built into a brief model and should be explicit from the outset.

**Transfer** occurs when needs exceed competence or setting, practitioner leaves, or a specialist service is indicated.

**Interruption** may result from illness, placement move, family crisis, transport failure, or school exclusion. It calls for active follow-up within policy.

**Dropout or unilateral ending** may signal dissatisfaction, improvement, practical barriers, shame, family conflict, or avoidance. Services should attempt respectful contact without harassment.

**Service-imposed ending** due to funding, age limits, non-attendance policies, or risk thresholds requires transparency, appeal routes where available, and transition planning. Calling it “client disengagement” does not erase institutional responsibility.

## 3. Beginning with the End in Mind

At intake, explain likely duration, review points, and what may trigger referral or ending. For time-limited work, name the number of sessions when known. For open-ended work, schedule periodic reviews.

Questions include:

- How will we know this has helped enough?
- What would you like to be different by our review?
- What concerns you about ending or changing counsellor?
- Who else should be involved in planning next steps?

This is not pessimism. A visible horizon helps clients use time and reduces dependence on the counsellor as the sole container of progress.

## 4. Evaluating Whether to End

Evaluation draws on symptoms, functioning, goals, relationships, participation, safety, client experience, and developmental trajectory. A young person may still have symptoms yet possess skills, supports, and a plan that make counselling no longer the right intensity. Another may score below a clinical threshold but remain isolated and unsafe.

Consider:

1. Have agreed goals been met or sufficiently progressed?
2. Can gains be maintained with ordinary supports?
3. Are risks understood and managed by a wider plan?
4. Has the young person internalised skills and help-seeking routes?

5. Are family and school responses supporting change?
6. Would continued sessions add value or create dependency?
7. Is another service better matched to remaining needs?

Ending from counselling does not mean ending all support. Step-down may involve mentoring, school check-ins, family support, peer groups, primary care, or planned booster sessions.

## 5. The Termination Phase

The final phase usually includes reviewing the original concerns, identifying changes and how they occurred, naming unresolved issues, rehearsing future challenges, updating safety and relapse plans, and deciding what records or summaries will be shared.

Young people may benefit from concrete artefacts: a coping card, letter to future self, timeline of progress, list of trusted adults, or summary in their own words. Such artefacts require consent and should not disclose sensitive material if lost or shared.

Booster sessions can be ethical when planned with purpose and limits. Open-ended “come back any time” promises should not be made where service rules do not permit them.

## 6. Further Reading for This Chapter

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- Norcross, J. C., Zimmerman, B. E., Greenberg, R. P., & Swift, J. K. (2017). Do all therapists do that when saying goodbye? *Psychotherapy, 54*(1), 66–75.
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## 7. Working with Reactions to Ending

Reactions may include relief, pride, indifference, grief, anger, anxiety, idealisation, devaluation, symptom return, or sudden requests for personal contact. Parents may feel gratitude, fear of losing support, or pressure to declare success. Practitioners may feel sadness, guilt, rescue urges, or relief; these require supervision.

Useful interventions include normalising mixed feelings, revisiting previous endings, allowing dissatisfaction to be spoken, and distinguishing ending counselling from ending care. Avoid interpreting every reaction as pathology. Sometimes anger is a proportionate response to an abrupt institutional decision.

Where deterioration occurs near ending, assess genuinely rather than assuming it is only separation protest. Extend or refer when risk warrants. At the same time, do not let every fluctuation indefinitely postpone a planned ending; build confidence in the wider support plan.

## 8. Practitioner Departure and Transfer

When a practitioner leaves, tell the young person as early as policy and certainty permit. Explain what will happen to records, who will receive a summary, whether a joint handover is possible, and what choices exist. Do not promise personal contact outside service systems.

A transfer summary should be concise and relevant: reason for referral, formulation, risk and safeguarding, interventions and response, current goals, medication or medical contacts where relevant, family and school arrangements, and what helps. Avoid sending entire process notes unless legally and clinically justified.

Joint handover sessions can help but may also overload. The young person should consent where possible and know what will be discussed.

## 9. Non-Attendance and Unplanned Endings

Services need humane non-attendance procedures. A missed appointment may indicate improvement, fear, transport problems, family conflict, forgetting, shame, or a confidentiality breach. Follow local policy, safeguarding duties, and the agreed contact method. Messages should not reveal sensitive information to whoever controls the phone.

Document attempts, close episodes transparently, and tell referrers what has occurred. Where risk is high, non-attendance may require escalation rather than administrative closure.

Practitioners should also examine service contributions to dropout: waiting times, inaccessible locations, culturally unsafe practice, poor alliance, inconvenient scheduling, or failure to involve the young person in goals.

## 10. Case Illustration: Ending Without Disappearing

**Presentation.** Tobi, seventeen, has completed twenty sessions for panic and school avoidance. Attendance and functioning have improved. The counsellor is leaving the school service in five weeks.

**Process.** The counsellor tells Tobi immediately rather than waiting until the final session. Tobi initially says, “Fine, I was going to stop anyway,” then misses the next appointment. A brief agreed text is sent without clinical detail. At the following session, Tobi acknowledges anger: adults “always go once things are working”.

The ending phase reviews the original panic diary, the mastery ladder, and the teacher arrangements that now belong to school rather than counselling. Tobi writes a future-self plan: early signs, first three steps, trusted adult, tutor check-in, and when to seek health care. A joint meeting with the school's pastoral lead confirms continuation of the entry and break plan. Tobi chooses not to transfer to another counsellor but knows the re-referral route.

At the final session, Tobi says the ending is “annoying but normal”. The phrase is clinically meaningful: separation has been survived without disappearance, punishment, or pretence that it does not matter.

## 11. Evaluation of Services and Practitioner Learning

Termination data should be examined by reason: planned completion, transfer, dropout, service capacity, moving, risk escalation, or dissatisfaction. Services should ask whether endings differ by ethnicity, disability, language, gender, care status, or deprivation. High completion rates may reflect selection rather than quality.

Client feedback at ending should include whether goals mattered, whether the young person felt respected, what was unhelpful, and what should change. Practitioner reflection can identify recurring difficulties with dependency, conflict, boredom, rescue, or avoidance of endings.

Evaluation is not only discharge measurement. It asks whether gains persist, whether referrals were accepted, whether school and family systems maintained changes, and whether re-referral reflects relapse, appropriate help seeking, or failure of step-down support.

## 12. Ethical Considerations

Ethical endings require honesty about time, competence, funding, and availability. Do not prolong treatment for practitioner income, training needs, research, or attachment. Do not terminate abruptly to punish non-compliance except where safety or professional boundaries require immediate action and policy supports it.

Post-termination contact should follow organisational and professional rules. Social media contact, personal messaging, gifts, and dual relationships require particular care with minors and former minor clients. Gratitude can be acknowledged warmly without creating private channels.

Closing records should distinguish outcome at ending from speculation about the future. Referrals should not exaggerate or minimise risk to secure acceptance. The young person should know what is being sent, subject to safeguarding exceptions.

## 13. Key Terms

**Key Terms at a Glance**

- **Termination** — The planned or unplanned conclusion of a counselling relationship.
- **Booster session** — A time-limited follow-up session to review maintenance and problem-solve setbacks.
- **Treatment dropout** — Client-initiated ending before agreed completion, whose meaning requires inquiry rather than assumption.
- **Step-down care** — Movement to a lower-intensity or more community-based form of support.
- **Handover** — Planned transfer of relevant information and responsibility between practitioners or services.
- **Relapse prevention** — Identification of warning signs, high-risk contexts, coping responses, and help-seeking routes.

## 14. Chapter Summary

Ethical endings are honest, prepared, evaluative, and connective. They return authorship to the young person, transfer useful knowledge to the systems that remain, and leave a clear route back to help. A good ending does not pretend the relationship did not matter; it demonstrates that mattering does not require permanence.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Why should termination be discussed at intake?
2. Distinguish planned termination, transfer, interruption, dropout, and service-imposed ending.
3. What outcomes besides symptom scores should inform ending?
4. How might deterioration near ending be understood without dismissing real risk?
5. Write a transfer summary structure that protects privacy.
6. What makes a booster session ethical or unethical?
7. How can services detect inequitable patterns in endings?

## 16. Practice Activities

**Practice and Application**

### Activity 54.1 — Ending letter

Draft a closing summary for a fictional adolescent in respectful language they could keep.

### Activity 54.2 — Departure conversation

Role-play telling a sixteen-year-old that you are leaving in four weeks, including likely anger and practical choices.

### Activity 54.3 — Ending audit

Create categories for auditing one year of case closures and identify what each category might reveal about service quality.

## 17. Annotated Reading Pathways

**For clinical process.** Termination research helps practitioners recognise common client and therapist reactions and avoid abrupt endings.

**For ethics.** Professional frameworks and local safeguarding policies should govern post-termination contact, record transfer, and non-attendance follow-up.

**For service evaluation.** Combine outcome scores with reason for ending, client experience, equity data, and follow-up where feasible.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Termination, Evaluation, and the Ethics of Endings** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is planned termination, relapse prevention, transfer, and the evaluation of outcome. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | “I know what this is and what may be shared.” | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | “Adults are not treating one setting as my whole life.” | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | “The problem has a pattern; I am not simply the problem.” | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | “This account fits, partly fits, or needs changing.” | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | “These goals are mine as well as the service's.” | Goal ratings and functional outcomes. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the

next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Daniel is fifteen and is seen in an integrated school and health team for conflict with parents about autonomy. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Daniel's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Daniel explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Daniel chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Daniel has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Daniel can describe the

pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?

- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Termination, Evaluation, and the Ethics of Endings

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Termination, Evaluation, and the Ethics of Endings** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Termination, Evaluation, and the Ethics of Endings** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

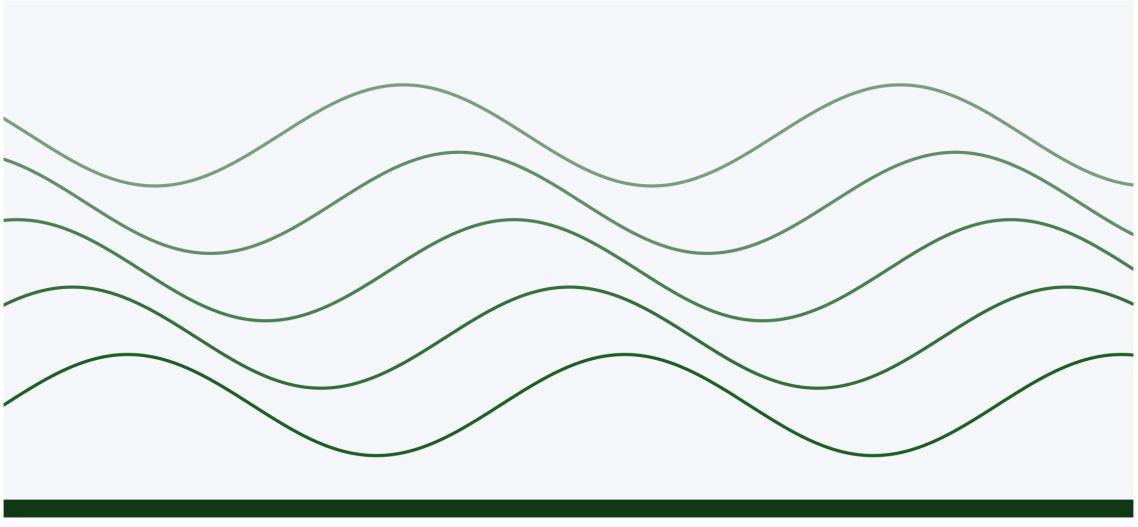
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Termination, Evaluation, and the Ethics of Endings**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Termination, Evaluation, and the Ethics of Endings to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 55

# Teletherapy and Technology in Counseling with Youth

Secure video practice, telehealth with minors, and digital ethics.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Compare video, telephone, text-based, asynchronous, app-supported, and blended counselling.
- Assess suitability and risk for remote work with children and adolescents.
- Establish privacy, identity, location, emergency, and safeguarding procedures.
- Adapt alliance building, play, family work, and outcome monitoring to digital settings.
- Evaluate apps, platforms, digital records, and artificial intelligence claims.
- Address digital equity, accessibility, online exploitation, and technology-related presenting problems.

## Chapter Overview

Technology is no longer an add-on to adolescent life or counselling. Young people form friendships, learn, play, seek help, experience bullying, explore identity, and manage distress through digital environments. Teletherapy can increase access for rural families, disabled young people, those with transport barriers, and students who find remote contact less intimidating. It can also create privacy risks, weaken observation, complicate safeguarding, and exclude those without devices, connectivity, or a safe room.

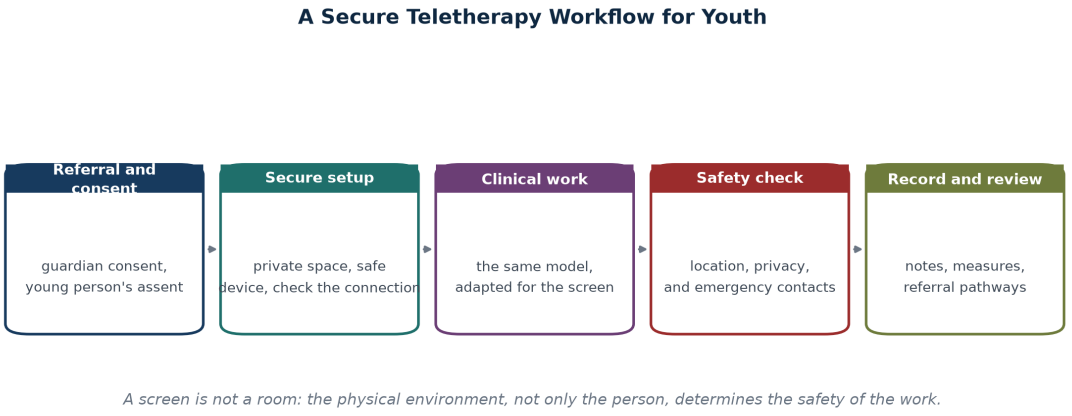



**Figure 55.47** A telehealth workflow: suitability assessment, consent, privacy and emergency checks, session adaptation, documentation, and review.

## 1. Modalities

**Synchronous video** most closely resembles in-person work and supports observation, family sessions, and visual materials.

**Telephone** may feel safer for some adolescents and avoids appearance concerns, but removes visual cues and can make silence harder to interpret.

**Live text or chat** can help young people who struggle to speak, but slows affect recognition and creates records requiring careful governance.

**Asynchronous messaging** permits reflection between contacts but risks boundary confusion, delayed crisis response, and expectations of continuous availability.

**Apps and digital tools** may support mood tracking, skills practice, exposure hierarchies, sleep routines, and outcome measures. They should supplement—not silently replace—clinical judgement.

**Blended care** combines remote and in-person sessions according to purpose: assessment or family work in person, brief reviews remotely, school consultation by secure video, and digital homework between sessions.

## 2. Evidence and Suitability

Research generally supports teletherapy effectiveness for several mental health conditions, including many anxiety and depression interventions, with outcomes often comparable to in-person treatment when appropriately delivered. Youth evidence is growing but varies by age, condition, platform, and degree of caregiver involvement.

Suitability is individual. Consider developmental level, attention, language, neurodivergence, sensory needs, risk, home privacy, technology access, family conflict, and the nature of interventions. Remote work may be unsuitable where there is acute risk without local support, severe dissociation, unsafe home monitoring, complex safeguarding investigation, or need for physical observation and intervention. Unsuitability is not permanent; reassess as circumstances change.

## 3. Before the First Remote Session

A technology-specific assessment and agreement should cover:

- identity verification and who else may be present;
- the young person's physical location at each session;
- privacy and interruptions;
- emergency contact and local crisis resources;
- a backup plan if connection fails;
- recording prohibition or consent;
- chat, email, and messaging boundaries;
- parent or carer involvement;
- school or service network security;
- what happens if abuse or risk is observed or disclosed.

The practitioner should also test camera, audio, accessibility features, captions, screen sharing, and data storage. Professional backgrounds and attire matter less than audibility, lighting, confidentiality, and freedom from household interruptions on the practitioner's side.

## 4. Privacy in Real Homes

A bedroom is not automatically private. Siblings may listen, parents may monitor, doors may lack locks, and devices may be shared. Ask what the young person can safely say and what should be avoided. Agree signals for “I can't speak freely”, options to switch to chat, or rescheduling without giving a reason that creates danger.

Headphones can increase privacy but reduce awareness of surroundings. The practitioner should periodically check who is nearby, especially before family conflict, sexuality, identity, abuse, or self-harm discussions.

School-based teletherapy requires a private room, not a corridor or shared office. Staff must know a session is occurring without receiving content.

## 5. Alliance and Presence Online

Digital presence requires explicit warmth. The practitioner names delays, looks toward camera periodically, summarises more often, checks understanding, and tolerates silence without filling it anxiously. Young people may prefer camera off at first; explore reasons rather than impose visibility unless assessment or safeguarding requires otherwise.

Creative adaptations include shared whiteboards, drawing applications, digital scaling, screen-shared worksheets with consent, music or images chosen by the young person, and movement breaks. The practitioner should not record, screenshot, or save shared material without agreement and a record-keeping rationale.

## 6. Further Reading for This Chapter


- American Telemedicine Association. *Practice Guidelines for Telemental Health with Children and Adolescents*.
- Shore, J. H. (Ed.). (2015). *Telepsychiatry and Health Technologies*.
- Hilty, D. M., et al. (2013). The effectiveness of telemental health: A 2013 review. *Telemedicine and e-Health, 19*(6), 444–454.
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## 7. Risk, Safeguarding, and Emergencies

At each session, confirm location and a contact number. Know local emergency and safeguarding routes for the young person's actual location, not merely the clinic's address. If risk escalates, keep the young person connected if safe, involve agreed emergency contacts, and follow local procedures.

Remote observation may reveal concerning backgrounds, voices, injuries, or another person directing answers. The practitioner should not investigate beyond their role where forensic or safeguarding specialists are required, but should document observations accurately and act promptly.

Online exploitation, image sharing, coercion, trafficking, and digital abuse may form part of the presenting problem. Preserve evidence according to specialist advice and law; do not ask a young person to forward illegal images to the counsellor. Involve safeguarding leads and appropriate authorities.

## 8. Family and Group Work Online

Remote family sessions require stronger choreography. Agree who speaks, how interruptions are handled, whether chat is used, and how a pause occurs. Separate breakout conversations need technical and confidentiality planning.

Group teletherapy requires screening, agreements about recording and screenshots, management of private messaging among members, and attention to who may be off-camera. Confidentiality cannot be guaranteed by the practitioner; this limit must be explicit.

Parents may value remote access, but a parent's presence in an adjoining room can change an adolescent's disclosure. Structure parent check-ins at the beginning or end rather than invisible monitoring throughout.

## 9. Digital Tools, Apps, and AI

Evaluate digital tools by asking:

1. What clinical problem does it address?
2. What evidence supports it for this age group?
3. What data are collected, where stored, and who can access or sell them?
4. Does it encourage unsafe comparison, compulsive use, or crisis delay?
5. Is it accessible and culturally appropriate?
6. Can the young person delete data and stop use?
7. Does it clearly direct users to human help in crisis?

Artificial intelligence chatbots and automated assessment tools require particular caution. They may simulate empathy without understanding, produce inaccurate advice, reinforce bias, or create dependency. They should not independently diagnose, manage risk, or replace human care for minors. Any organisational use requires validation, transparency, safeguarding design, data governance, and accountable human oversight.

## 10. Technology-Related Presenting Problems

Digital life can be context, resource, symptom, and stressor at once. Assessment should avoid moral panic and ask functionally:

- What does the young person gain online—belonging, mastery, escape, identity, income, creativity?
- What is displaced—sleep, movement, study, family contact, offline friendship?
- What harms occur—bullying, comparison, coercion, gambling-like mechanics, pornography, misinformation, contact risks?
- What control exists—algorithms, notifications, peer expectations, fear of missing out?

- What change does the young person want?

Interventions may involve sleep routines, notification design, platform safety, family agreements, alternative belonging, school anti-bullying action, and treatment of underlying anxiety, depression, loneliness, or neurodevelopmental needs. Confiscation alone rarely teaches regulation and may remove support networks.

## 11. Case Illustration: Therapy from a Parked Car

**Presentation.** Sena, seventeen, is referred for anxiety. She joins video sessions from a parked car outside home because her younger siblings share her room and her mother listens at doors. The practitioner initially treats the car as a privacy solution.

**Assessment.** Sena is not licensed to drive alone to a safer place, the car is hot, sessions are rushed before a parent returns, and she cannot use grounding movement. On one occasion she becomes distressed while a parent is knocking on the window.

**Reformulation.** Remote access solved travel but not privacy or safety. The service arranges a hybrid plan: fortnightly in-person sessions at a youth centre, brief remote check-ins only for practical planning, and a written agreement that sensitive work occurs in person. Sena's mother receives general guidance on privacy without session content.

The case shows that technological convenience must be tested against embodiment, safety, confidentiality, and the young person's actual environment.

## 12. Equity and Accessibility

Digital exclusion can mirror social inequality. Services should ask about devices, data costs, connectivity, shared space, disability access, language, and digital literacy. Offer telephone or in-person alternatives, captioning, interpreters, low-bandwidth options, and assistance with setup.

Accessibility includes screen-reader compatibility, keyboard navigation, visual contrast, reduced motion, and alternatives for young people who find eye contact or camera self-view distressing. Accessibility is not an enhancement; it is equitable care.

## 13. Key Terms

**Key Terms at a Glance**

- **Teletherapy** — Counselling delivered through telecommunications technology.
- **Blended care** — Planned combination of in-person, remote, and digital-supported care.
- **Asynchronous contact** — Communication exchanged with time delays, such as secure messaging.
- **Digital equity** — Fair access to devices, connectivity, skills, privacy, and accessible design.
- **Online exploitation** — Coercive or abusive use of digital communication for sexual, financial, or other exploitation.
- **Data governance** — Rules and accountability for collecting, storing, accessing, sharing, and deleting data.

## 14. Chapter Summary

Technology can widen the counselling room or quietly shrink it. Ethical teletherapy begins before login: suitability, location, privacy, emergency arrangements, accessibility, and boundaries. During sessions, the practitioner adapts presence and methods. Afterward, records and data require the same care as the conversation itself.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Compare video, telephone, chat, asynchronous messaging, and blended care for adolescents.
2. List ten checks required before a first remote session.
3. How can a private home still be an unsafe therapy setting?
4. What should happen if a young person's location changes to another jurisdiction?
5. Evaluate a fictional mental health app using the seven questions in this chapter.
6. Why should AI chatbots not independently manage adolescent risk?
7. Identify three digital equity adjustments a service can make.

## 16. Practice Activities

**Practice and Application**

### Activity 55.1 — Remote-session agreement

Draft a one-page agreement covering privacy, location, recording, messaging, emergencies, and connection failure.

### Activity 55.2 — Modality match

For panic treatment, family conflict, grief, and social skills practice, choose a modality and defend the choice.

### Activity 55.3 — Privacy rehearsal

Role-play discovering that a parent is listening outside the door during a video session with a sixteen-year-old.

## 17. Annotated Reading Pathways

**For guidelines.** Use current national and organisational telemental health standards because technology and law change quickly.

**For evidence.** Read youth-specific systematic reviews and distinguish guided digital interventions from unguided apps.

**For ethics.** Combine data protection, safeguarding, digital rights, and professional boundary guidance before adopting any new platform.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Teletherapy and Technology in Counseling with Youth** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is secure video practice, telehealth with minors, and digital ethics. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | "I know what this is and what may be shared." | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | "Adults are not treating one setting as my whole life." | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | "The problem has a pattern; I am not simply the problem." | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | "This account fits, partly fits, or needs changing." | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | "These goals are mine as well as the service's." | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | "I have a way to begin, not just an instruction." | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | "The work is challenging but understandable and stoppable." | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | "This is not only something I do in a counselling room." | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | "A setback leads to inquiry, not blame." | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | "Adults are changing some conditions too." | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | "I can explain what helps and why." | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | "Ending is planned; I am not disappearing from care." | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Nia is fifteen and is seen in a school counselling service for shame after academic failure. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Nia's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Nia explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated

questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Nia chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Nia has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Nia can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Teletherapy and Technology in Counseling with Youth

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Teletherapy and Technology in Counseling with Youth** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Teletherapy and Technology in Counseling with Youth** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

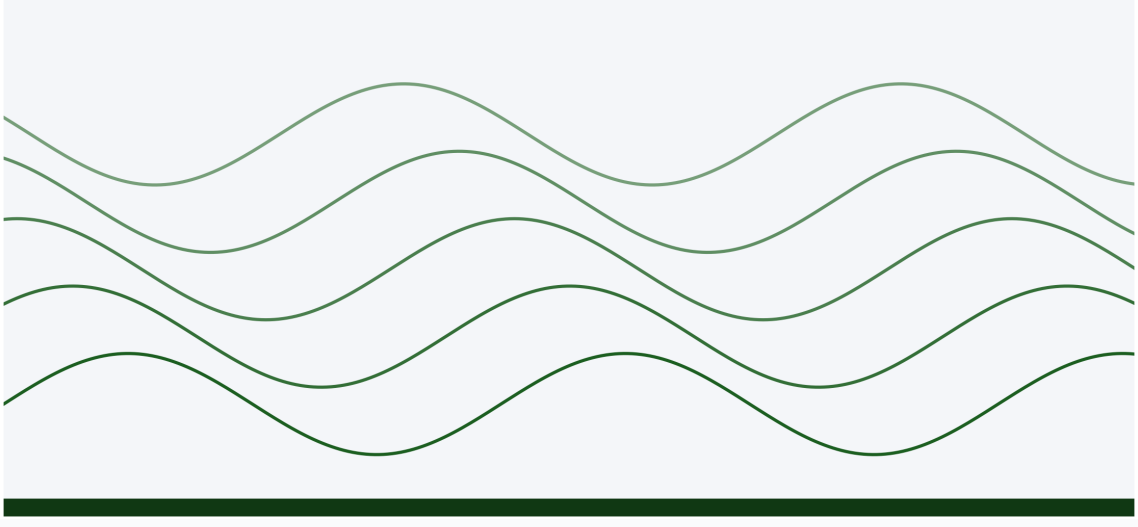
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Teletherapy and Technology in Counseling with Youth**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Teletherapy and Technology in Counseling with Youth to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 56

# School Counseling Practice and Consultation

The school counselor's role, the consultation model, and the ethics of working in an institutional setting.



## Learning Objectives

By the end of this chapter, the student should be able to:

- Describe the school counsellor's roles in direct counselling, consultation, prevention, advocacy, coordination, and systems change.
- Distinguish counselling from discipline, attendance enforcement, and behaviour management.
- Build ethical confidentiality practices within a school.
- Consult with teachers and families using collaborative problem-solving.
- Develop tiered supports within a multi-tiered system.
- Respond to crises, safeguarding concerns, and school-wide trauma.
- Evaluate school counselling outcomes and equity.

## Chapter Overview

Schools are where adolescents spend much of their social lives, encounter evaluation and belonging, form peer relationships, and become visible to adults outside the family. School counselling therefore cannot be only a quiet room and a waiting list. It includes brief individual work, groups, classroom and whole-school prevention, consultation with teachers, family collaboration, crisis response, referral, advocacy, and examination of the systems that produce distress.

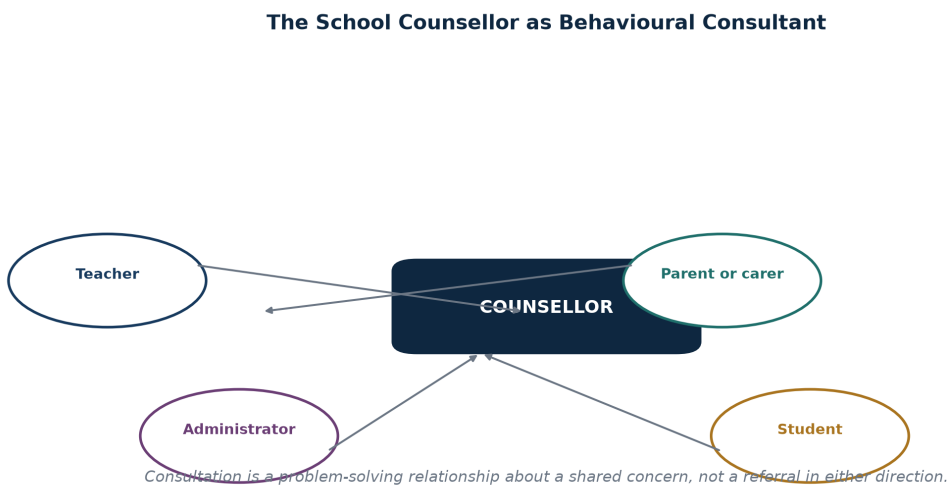



**Figure 56.48** School consultation: counsellor, teacher, family, and student perspectives converge around a shared problem definition and classroom plan.

## 1. The Distinctive School Context

School counsellors work inside the institution that may be both resource and stressor. A counsellor can help a student cope with examination pressure while also questioning public ranking practices. They can support behaviour plans while resisting the use of counselling as punishment. They can share functional information with teachers while protecting confidential disclosures.

This dual position requires role clarity. At the beginning of work, explain:

- what counselling is and is not;
- who can refer and why;
- what remains confidential;
- what must be shared for safety;
- what summaries teachers may receive;
- how counselling differs from discipline;
- how students can request or decline help, subject to local policy and safeguarding.

Without clarity, students may experience counselling as surveillance and teachers may expect reports the counsellor cannot ethically provide.

## 2. Tiered Support

A multi-tiered system organises support by intensity.

**Tier 1: universal.** Safe climate, belonging, social-emotional learning, anti-bullying systems, mental health literacy, staff training, accessible routines, and student voice.

**Tier 2: targeted.** Small groups, mentoring, brief interventions, attendance support, family check-ins, and skills programmes for students with emerging needs.

**Tier 3: intensive.** Individual counselling, coordinated care, specialist referral, safety planning, and personalised school adjustments.

Tiering should be a route to help, not a queue that requires failure at each level before support increases. Screening and teacher referral must be monitored for bias: quiet internalising students, disabled students, racialised students, LGBTQ+ students, carers, and students whose families distrust services may be identified differently.

## 3. Direct Counselling in Schools

School counselling is often brief, interrupted by timetables, examinations, staff absence, and holidays. Practitioners need focused assessment, clear goals, session-by-session review, and planned endings between terms.

Confidentiality in corridors matters. Appointment slips, room location, teacher messages, and being collected from class can disclose service use. Systems should allow discreet scheduling.

Evidence-informed school work may draw on CBT, behavioural activation, motivational interviewing, solution-focused practice, IPT-A, trauma-informed stabilisation, family consultation, and group interventions. Choice depends on formulation and competence, not simply available time.

## 4. Consultation

Consultation is indirect service: the counsellor helps a teacher, parent, or team change the environment or interaction around a student. It is not giving orders from superior expertise.

A collaborative consultation sequence includes:

1. establish purpose and confidentiality;
2. define the concern in observable terms;
3. gather context and previous responses;
4. consider student perspective and function;
5. generate options;
6. choose small feasible actions;
7. assign responsibilities;
8. review data and revise.

Teacher consultation must respect workload. A plan requiring six adults to perform ten steps perfectly will fail. One change in instruction delivery, seating, feedback privacy, or break access may be more effective.

## 5. Family Collaboration

Families know routines, history, culture, strengths, and barriers invisible at school. Collaboration begins with listening rather than reporting failures. Interpreters, flexible meeting times, remote options, and accessible documents are equity practices.

Where family and school accounts conflict, the counsellor avoids becoming a messenger of blame. Joint meetings need purpose, preparation, student voice, and agreement about what will be recorded.

## 6. Further Reading for This Chapter

- American School Counselor Association. (2022). *ASCA National Model* (4th ed.).
- Stone, C. B. (2022). *Ethics and Law for School Counselors* (5th ed.).
- Capuzzi, D., & Stauffer, M. D. (Eds.). (2016). *Counseling and Psychotherapy: Theories and Interventions*.
- Baker, S. B., & Gerler, E. R. (2008). *School Counseling for the Twenty-First Century* (5th ed.).
- Brown, D., & Trusty, J. (2005). *Designing and Leading Comprehensive School Counseling Programs*.

## 7. Crisis and Postvention

School crises include student death, suicide, violence, accidents, community disasters, and public incidents amplified by social media. Counsellors help coordinate factual communication, support for affected groups, identification of vulnerable students, family liaison, staff support, and referral.

Postvention after a suicide requires specialist guidance: avoid glamourising details, monitor contagion risk, support grief, communicate safely, and provide sustained follow-up. Rumours and online material should be addressed without repeating harmful content unnecessarily.

Crisis work also includes returning to ordinary routines. Predictability is regulating. Memorial practices should be inclusive, voluntary, culturally sensitive, and consistent with guidance.

## 8. Safeguarding and Multi-Agency Work

School counsellors often receive disclosures because they are accessible and trusted. They must know local safeguarding procedures, record accurately, avoid investigative interviewing beyond their role, and communicate promptly with designated leads.

Multi-agency work requires consent where possible, clear purpose, minimum necessary information, and attention to the young person's experience of meetings. A student should know who will attend, what will be discussed, and how their views will be represented.

Advocacy may involve special educational needs assessment, disability accommodations, exclusion prevention, alternative provision, housing or benefits referral, health appointments, and protection from bullying or discrimination.

## 9. Behaviour, Attendance, and Belonging

The counsellor's contribution to behaviour and attendance is formulation, not merely consequence delivery. Useful questions include: What skill is missing? What is being avoided or obtained? Where does the behaviour not occur? What adult responses help? What curriculum, sensory, language, health, transport, or peer factors operate?

Restorative approaches can repair harm where participation is voluntary and safety permits. They should not force victims to confront perpetrators or substitute for safeguarding.

Attendance work combines early identification, family partnership, practical barriers, school belonging, health, graded return, and monitoring. Public attendance shaming and purely punitive approaches often deepen avoidance.

## 10. Case Illustration: The Counsellor Who Refused the Report

**Situation.** A head of year asks the counsellor to report whether a fifteen-year-old “admitted” leaving school grounds and to recommend a sanction.

**Response.** The counsellor explains the role boundary: counselling content cannot be converted into disciplinary evidence, except where safeguarding or agreed information sharing applies. The counsellor offers consultation instead: review supervision patterns, reasons students leave at lunch, safe spaces, and a support plan based on information the student agrees to share.

**Consultation.** Data show most departures occur after a crowded lunch period and before double science. The student, consulted separately with consent, identifies noise, hunger due to long queues, and anxiety about entering a lesson late after lunch. The school adjusts lunch access, creates a quieter eating space, and permits direct entry to science for two weeks. The counsellor works individually on panic and planning.

The refusal protects trust while still helping the institution solve its problem.

## 11. Programme Evaluation and Advocacy

School counselling should evaluate:

- access and waiting time;
- student-defined goals and experience;
- symptoms and functioning;
- attendance, participation, and attainment where relevant;
- equity of referral and outcome;
- teacher and family consultation impact;
- crisis response and referral completion.

Data should be reported in aggregate and used to improve systems, not to pressure counsellors into selecting easy cases. Student voice should inform service design, including room location, referral routes, session length, digital access, and confidentiality communication.

Advocacy arises from patterns: repeated bullying in one location, inaccessible homework platforms, exclusion disparities, lack of interpreters, or timetable structures that prevent sleep and eating. The counsellor brings data, student perspective, and feasible proposals to leadership.

## 12. Ethics and Professional Identity

School counsellors manage dual loyalties to student, family, school, profession, and law. Written role statements, supervision external to line management where possible, record separation, and clear information-sharing agreements reduce conflict.

Counsellors should not conduct therapy with students they also discipline, grade, or investigate. Where small schools make overlap unavoidable, disclose limits and seek supervision and alternative referral options.

Caseload and time must permit safeguarding and crisis duties. A service measured only by number of sessions may neglect prevention and consultation where population impact is greater.

## 13. Key Terms

**Key Terms at a Glance**

- **Consultation** — Collaborative indirect service that helps adults alter environments or interactions around a young person.
- **Tiered support** — Organisation of universal, targeted, and intensive interventions.
- **Postvention** — Coordinated support after a death or crisis to reduce harm and support recovery.
- **Role clarity** — Explicit definition of the counsellor's functions, limits, and information-sharing practices.
- **Student voice** — Meaningful participation of students in decisions and service evaluation.
- **Restorative practice** — Voluntary processes focused on understanding harm, accountability, and repair.

## 14. Chapter Summary

School counselling is both intimate and institutional. It protects a confidential relationship while changing classrooms, corridors, policies, and partnerships around the student. Its credibility depends on refusing incompatible roles, consulting collaboratively, and using evidence to advocate for environments in which fewer students need rescue.

## 15. Review and Discussion Questions

**Review and Discussion Questions**

1. Distinguish counselling, consultation, prevention, coordination, and advocacy in a school.
2. Why should counselling not function as discipline?
3. Design a three-tier response to exam anxiety.
4. Write a confidentiality explanation suitable for a thirteen-year-old and their teacher.
5. Apply the eight-step consultation sequence to repeated classroom exit.
6. What data would reveal inequitable access to school counselling?
7. How should a counsellor respond to a request for session content to support a sanction?

## 16. Practice Activities

**Practice and Application**

### Activity 56.1 — Role statement

Draft a 200-word school counselling role statement for a student handbook.

### Activity 56.2 — Consultation plan

Use the collaborative sequence to address a student who leaves one teacher's lessons but not others.

### Activity 56.3 — Tier map

Map a school's response to bullying across universal, targeted, and intensive levels.

## 17. Annotated Reading Pathways

**For comprehensive programmes.** The ASCA model is influential but should be adapted to local governance and professional standards.

**For ethics.** Stone's work is valuable on confidentiality, records, court involvement, and school-specific dilemmas.

**For practice.** Combine school counselling texts with implementation science, special education, attendance research, and child safeguarding guidance.

## Practice Laboratory and Extended Worked Example

This laboratory turns **School Counseling Practice and Consultation** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is the school counselor's role, the consultation model, and the ethics of working in an institutional setting. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

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| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | “I have a way to begin, not just an instruction.” | Observed practice, mastery, and confidence ratings. |

| Phase | Practitioner task | Young person's experience | Evidence to review |
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| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | “The work is challenging but understandable and stoppable.” | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | “This is not only something I do in a counselling room.” | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | “A setback leads to inquiry, not blame.” | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | “Adults are changing some conditions too.” | Meeting actions, school adjustments, and referral outcomes. |
| 11. Cons olidate | Name the strategies, relationships, and environmental changes that produced improvement. | “I can explain what helps and why.” | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | “Ending is planned; I am not disappearing from care.” | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support,

timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Marcus is fifteen and is seen in a community adolescent clinic for school avoidance and rising panic. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Marcus's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Marcus explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Marcus chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Marcus has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Marcus can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point

at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for School Counseling Practice and Consultation

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **School Counseling Practice and Consultation** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **School Counseling Practice and Consultation** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **School Counseling Practice and Consultation**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into

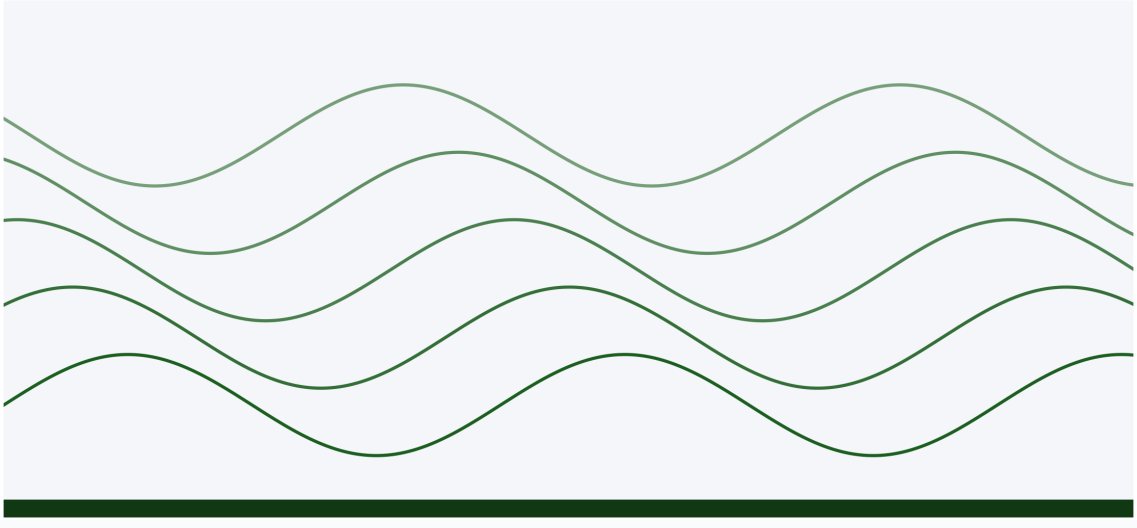
clinical governance.

**Written consolidation.** In 250 words, explain School Counseling Practice and Consultation to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 57

# Teaching CAC. 701: Session Plans and Facilitation

A complete teaching sequence for the course, including delivery notes, activities, and assessment alignment.



## Learning Objectives

By the end of this chapter, the lecturer should be able to:

- Translate the CAC. 701 syllabus into a coherent sixteen-week teaching sequence.
- Construct session plans with outcomes, preparation, teaching activities, assessment, and resources.
- Facilitate discussion of counselling theory without reducing it to memorisation.
- Use case vignettes, role play, observation, writing, and digital resources safely.
- Support students with varied academic, linguistic, disability, and professional backgrounds.
- Assess participation and contribution without rewarding disclosure of personal trauma.
- Evaluate and revise teaching using student feedback and assessment evidence.

## Chapter Overview

Teaching counselling theories is not the delivery of a museum of famous names. Students must learn to think with theories: to notice what a theory sees, what it misses, how it explains change, what interventions follow, what evidence supports it, whom it fits, and where it may harm. A CAC. 701 session should therefore alternate explanation, demonstration, comparison, practice, critique, and reflection.

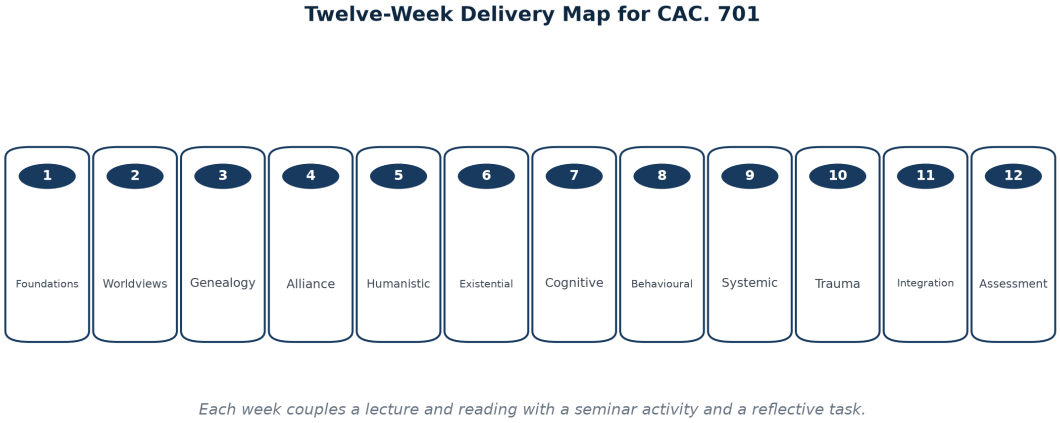



**Figure 57.49** A course map for CAC. 701 linking foundational concepts, major theory families, youth applications, integration, assessment, and professional practice.

This chapter offers a teaching architecture rather than a script. Lecturers should adapt examples, pacing, readings, and assessments to institutional requirements and student needs while preserving constructive alignment: outcomes, activities, and assessment must point in the same direction.

## 1. Course Design Principles

**Start with clinical problems.** Students remember theories better when each is introduced through a human situation. Begin with a brief vignette, ask what needs explaining, then present the theory as one disciplined way of seeing.

**Compare early and often.** Comparison prevents brand loyalty. Ask how behavioural, cognitive, humanistic, family, trauma-informed, and cultural accounts differ in cause, change, role of counsellor, and evidence.

**Teach evidence with concepts.** A theory should not be presented first as timeless truth and later as research topic. Include evidence, limitations, and populations studied as part of the theory's identity.

**Practise safely.** Skills laboratories require consent, alternatives to personal disclosure, clear roles, stop rules, and debriefing. Students should practise on fictional or consented material, not be required to reveal trauma to pass.

**Make youth central.** Every adult-origin theory must be interrogated for developmental fit: cognition, language, autonomy, family dependence, school context, consent, and power.

**Model cultural humility.** Teaching examples should vary across family forms, languages, disability, class, migration, gender, sexuality, and community contexts without turning any group into a spectacle.

## 2. Sixteen-Week Sequence

A possible sequence is:

1. Why theory matters; course map; formulation as disciplined seeing.
2. Scientific, philosophical, and professional foundations.
3. Worldviews, common factors, alliance, and evidence appraisal.
4. Developmental and cultural foundations for youth counselling.
5. Person-centred, existential, Gestalt, and focusing approaches.
6. Transactional analysis, internal family systems, and parts language.
7. Cognitive model, REBT, schema concepts, and youth CBT.
8. ACT, DBT skills, mindfulness, and third-wave debates.
9. Behavioural principles, functional assessment, exposure, and contingencies.
10. Behavioural activation, school and clinic systems.
11. Family systems: Bowen, structural, and strategic approaches.
12. Attachment, solution-focused, and narrative practice.
13. Emotion-focused, trauma-informed, EMDR, stabilisation, and somatic practice.
14. IPT-A, mentalization, motivational interviewing, and social cognitive theory.
15. Adolescent development, psychodynamic, multicultural, feminist, intersectional, LGBTQ+ affirmative, resilience, and ecological practice.
16. Integration, case conceptualisation, assessment, ethics of endings, teletherapy, school consultation, and capstone formulation.

This sequence compresses substantial material in later weeks. Institutions with more time may divide weeks 14–16 into separate units. The logic is cumulative: foundations, relationship, major mechanisms, systems, development and culture, then integration and professional use.

## 3. Anatomy of a Three-Hour Session

A three-hour postgraduate session might use the following rhythm:

**Opening and retrieval (15 minutes).** A low-stakes question connecting to the previous week.

**Clinical problem (15 minutes).** A short case vignette read aloud or shown in text; students identify what needs explanation.

**Concept teaching (35 minutes).** Core theory, key terms, mechanisms, and a diagram.

**Demonstration (25 minutes).** Lecturer demonstration, recorded session excerpt with permission, transcript analysis, or live structured role play.

**Break (10 minutes).**

**Application laboratory (40 minutes).** Triads practise with fictional material; observer uses a structured feedback form.

**Evidence and critique (20 minutes).** Trial or review summary, limitations, cultural questions, populations missing.

**Formulation synthesis (15 minutes).** Students write what the theory changes in a treatment plan.

**Exit assessment (5 minutes).** One concept learned, one question remaining, one application to youth practice.

Not every week needs every element. Predictable rhythm reduces anxiety, while variation in tasks maintains attention.

## 4. Facilitation Moves

Useful lecturer moves include:

- asking for observations before interpretations;
- inviting competing theoretical readings of the same event;
- writing student language verbatim before translating into theory;
- pausing after questions long enough for thought;
- asking who benefits and who may be harmed by an intervention;
- distinguishing evidence, clinical tradition, and personal preference;
- summarising disagreement accurately;
- returning to the young person's voice.

Lecturers should avoid rewarding the most confident speaker as the most competent thinker. Written responses, pairs, anonymous digital polls, and small groups distribute participation.

## 5. Further Reading for This Chapter


- Biggs, J., & Tang, C. (2011). *Teaching for Quality Learning at University* (4th ed.).
- Brookfield, S. D. (2017). *Becoming a Critically Reflective Teacher* (2nd ed.).
- Fink, L. D. (2013). *Creating Significant Learning Experiences* (2nd ed.).

- Ambrose, S. A., et al. (2010). *How Learning Works*.
- Nilson, L. B. (2016). *Teaching at Its Best* (4th ed.).

# 6. Session Plans for Selected Units

## Session A: Functional assessment

**Outcomes.** Students can operationalise a behaviour, generate at least three functional hypotheses, and identify data that would discriminate among them.

**Preparation.** Read Chapters 24 and 26; bring an anonymised ABC observation from practice or use the provided fictional record.

**Activities.** Ten-minute concept review; lecturer models faulty versus testable hypotheses; groups analyse a school incident log; each group designs one observation and one intervention; plenary compares ethical safeguards.

**Assessment.** Exit ticket requiring one operational definition and one disconfirming observation.

## Session B: Family systems

**Outcomes.** Students can draw a structural map, identify triangles, and separate structural from interactional formulation.

**Preparation.** Read Chapters 30–33.

**Activities.** Case reading; small-group mapping; lecturer-led comparison of Bowenian, structural, and strategic plans; practice a family opening statement; critique cultural assumptions.

**Assessment.** Short formative map submitted for feedback, not grading of artistic quality.

## Session C: Trauma-informed practice

**Outcomes.** Students can identify state, phase, safety requirements, and inappropriate timing of trauma processing.

**Preparation.** Read Chapters 38 and 40.

**Activities.** State-recognition exercise using fictional transcripts; phase placement; grounding demonstration with opt-out; design a school environment audit; discuss the difference between trauma-informed care and trauma-specific treatment.

**Assessment.** Written phase justification with criteria for moving forward and backward.

## Session D: Integration capstone

**Outcomes.** Students can produce a coherent, sequenced, culturally responsive plan and defend omissions as well as inclusions.

**Activities.** Complex case distributed in stages; groups revise formulation as new information arrives; cross-group peer review; lecturer models supervision questions.

**Assessment.** Draft capstone formulation for formative feedback before final submission.

## 7. Role Play and Demonstration Ethics

Role play is a laboratory, not group therapy. The lecturer must:

- use fictional cases by default;
- obtain consent for roles and recording;
- permit observer or writer roles;
- set stop signals;
- prohibit use of classmates' real trauma;
- debrief role occupants out of character;
- give feedback on observable skills, not personality;
- provide alternatives for students for whom a scenario is personally activating.

Demonstrations should be interrupted for teaching points rather than performed as virtuoso displays. If a demonstration fails, analyse it. Students learn more from repair than perfection.

## 8. Inclusive Teaching

Accessibility begins before requests. Provide slides and readings in accessible formats, caption videos, describe diagrams, allow assistive technology, avoid colour-only meaning, and state assessment criteria plainly. Offer multiple ways to participate: speech, writing, anonymous response, small group, and asynchronous forum.

Language support includes glossaries, pre-reading questions, and permission to discuss concepts in a stronger language before presenting in academic English where institutional rules permit. International students should not be treated as deficits when their experience exposes the cultural limits of a theory.

Attendance flexibility must balance professional formation with disability, caring, work, health, and transport realities. Participation grades based solely on speaking frequency are inequitable.

## 9. Difficult Classroom Moments

Students may disclose personal trauma, challenge a theory on cultural grounds, defend a prohibited practice, disclose risk, or become distressed. The lecturer should acknowledge, protect privacy, avoid turning disclosure into teaching spectacle, and follow safeguarding routes.

Disagreement is valuable. Distinguish critique of ideas from attacks on people. Ask speakers to identify evidence and implications. Where discussion involves identities represented in the room, do not appoint any student as spokesperson for a group.

Assessment integrity requires clear policy on collaboration, generative AI, plagiarism, and use of client material. Students must anonymise practice examples and never submit identifiable client information.

## 10. Evaluating Teaching

Evaluation should combine student feedback, peer observation, assessment patterns, and lecturer reflection. If many students confuse two concepts, teaching may need restructuring rather than students simply being

told to study harder.

Track whether case examples remain diverse, whether skills practice produces competence, whether feedback is timely, and whether assessment tasks align with outcomes. Revise one or two elements at a time so changes can be understood.

## 11. Key Terms

**Key Terms at a Glance**

- **Constructive alignment** — Coherence among outcomes, learning activities, and assessment.
- **Formative assessment** — Assessment used to guide learning before final judgement.
- **Retrieval practice** — Actively recalling prior learning to strengthen understanding.
- **Skills laboratory** — Structured practice with fictional material, observation, feedback, and debriefing.
- **Inclusive teaching** — Design that anticipates varied access, language, disability, and participation needs.
- **Capstone formulation** — A culminating case conceptualisation integrating theory, development, culture, evidence, and planning.

## 12. Chapter Summary

Teaching CAC. 701 is itself an exercise in formulation. The lecturer identifies what students need to notice, designs experiences that make theory usable, observes misunderstanding without shame, and revises. A successful course does not produce loyalty to one school. It produces practitioners who can explain their choices, recognise limits, and keep the young person—not the theory—at the centre.

## 13. Review and Discussion Questions

**Review and Discussion Questions**

1. Why should theory teaching begin with clinical problems?
2. Design a three-hour session on exposure that includes evidence and ethics.
3. What safeguards distinguish a skills laboratory from group therapy?
4. How can participation be assessed equitably?
5. Identify one point where a lecturer's preferred theory could bias teaching.
6. What student assessment evidence would lead you to revise a session?
7. Explain constructive alignment using the functional assessment session plan.

## 14. Practice Activities

**Practice and Application**

### Activity 57.1 — Session design

Create a 90-minute session plan for narrative therapy with outcomes, preparation, activities, assessment, and accessibility notes.

### Activity 57.2 — Demonstration debrief

Write a debrief script for a role play in which the “counsellor” accidentally pressures a student actor to disclose.

### Activity 57.3 — Alignment audit

Take one course outcome and trace it through readings, activities, formative work, and final assessment. Identify any break in alignment.

## 15. Annotated Reading Pathways

**For course design.** Biggs and Tang and Fink help align outcomes, activities, and assessment.

**For facilitation.** Brookfield supports critical reflection on power and student perception.

**For learning science.** Ambrose and colleagues provide practical principles on prior knowledge, organisation, motivation, feedback, and self-directed learning.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Teaching CAC. 701: Session Plans and Facilitation** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is a complete teaching sequence for the course, including delivery notes, activities, and assessment alignment. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements;

others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | "I know what this is and what may be shared." | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | "Adults are not treating one setting as my whole life." | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | "The problem has a pattern; I am not simply the problem." | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | "This account fits, partly fits, or needs changing." | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | "These goals are mine as well as the service's." | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | "I have a way to begin, not just an instruction." | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | "The work is challenging but understandable and stoppable." | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | "This is not only something I do in a counselling room." | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | "A setback leads to inquiry, not blame." | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | "Adults are changing some conditions too." | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | "I can explain what helps and why." | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | "Ending is planned; I am not disappearing from care." | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable, and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Leah is fifteen and is seen in a family support service for repeated classroom escalation. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Leah's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner

first establishes safety, consent, and context. Leah explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home, concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Leah chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Leah has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Leah can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.

- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.
- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Teaching CAC. 701: Session Plans and Facilitation

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Teaching CAC. 701: Session Plans and Facilitation** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's

goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Teaching CAC. 701: Session Plans and Facilitation** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

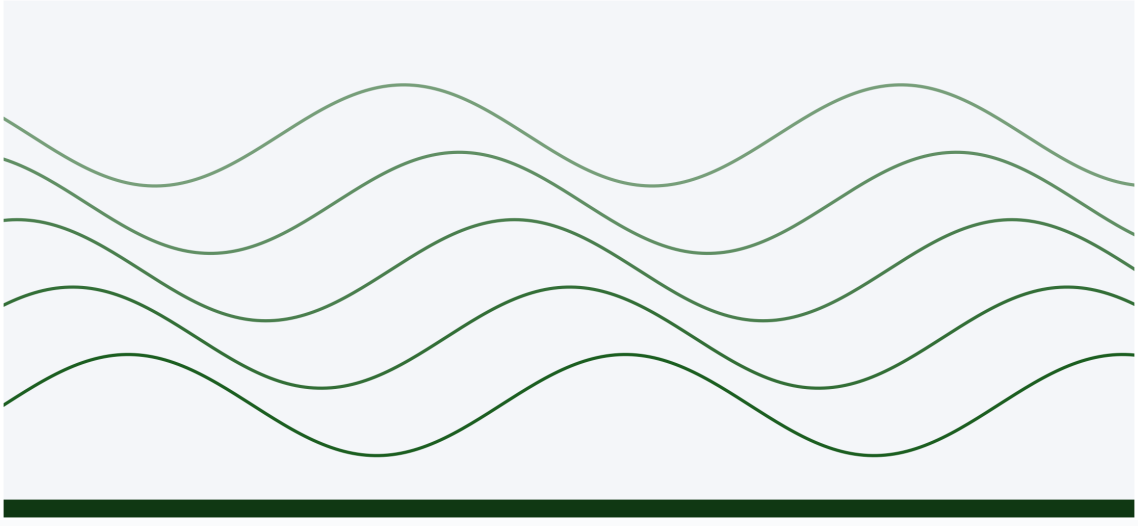
**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Teaching CAC. 701: Session Plans and Facilitation**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Teaching CAC. 701: Session Plans and Facilitation to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER 58

# Assessment Blueprint, Assignments, and Rubrics

Course-level assessment design, major assignments, grading rubrics, and the assurance of academic integrity.



## Learning Objectives

By the end of this chapter, the student and lecturer should be able to:

- Map CAC. 701 assessments to course outcomes and programme expectations.
- Design valid, reliable, fair, and feasible assignments for theory learning.
- Write transparent marking criteria and rubrics.
- Assess case conceptualisation, evidence appraisal, skills demonstration, reflection, and professional judgement.
- Provide feedback that students can use in subsequent work.
- Apply academic integrity, accessibility, moderation, and appeals principles.
- Use assessment data to improve teaching and demonstrate course quality.

## Chapter Overview

Assessment tells students what the course truly values. If lectures praise cultural humility, developmental reasoning, and client voice but examinations reward only memorised names and dates, students will memorise names and dates. The CAC. 701 blueprint therefore assesses theory as professional capability: seeing, formulating, choosing, justifying, practising, evaluating, and revising.

**Course Assessment Blueprint for CAC. 701**

| Learning outcome | Teaching activity | Assessment evidence | Weight |
|---|---|---|---|
| Explain and compare major theories | Lectures, seminar comparison tables | Theory comparison paper | 20% |
| Apply theory to a child or adolescent case | Case formulation seminar | Formulation assignment | 25% |
| Evaluate the evidence for a model | Critical reading seminar | Annotated bibliography | 15% |
| Demonstrate core counselling skills | Role-play and skills laboratory | Skills demonstration | 15% |
| Reflect on ethical and cultural practice | Ethics case clinic | Reflective journal and viva | 15% |
| Design an evidence-based treatment plan | Planning workshop | Treatment plan submission | 10% |

*Every assessment task maps to a stated learning outcome and to at least one course objective.*

**Figure 58.50** An assessment blueprint mapping outcomes to essays, case formulation, evidence appraisal, skills practice, reflection, and examination.

## 1. Blueprint Principles

A defensible blueprint states:

- course learning outcomes;
- programme and professional competencies addressed;
- assessment tasks and weighting;

- timing and formative preparation;
- criteria and standards;
- moderation and feedback arrangements;
- accessibility and integrity requirements.

Validity asks whether the task measures the intended capability. Reliability asks whether judgements are consistent. Fairness asks whether irrelevant factors—handwriting, accent, confidence, cultural style, disability, technology access—distort performance. Feasibility asks whether students and markers can complete the work to the stated standard.

No single task can assess everything well. The course therefore combines written analysis, case formulation, evidence appraisal, observed practice, reflective commentary, and invigilated or time-bound synthesis where required by institutional rules.

## 2. Proposed Assessment Pattern

Subject to institutional approval, a balanced pattern is:

| Assessment | Weight | Primary capability |
|---|---|---|
| Theory comparison essay | 20% | Conceptual understanding and critical comparison |
| Evidence appraisal brief | 15% | Research literacy and application to youth |
| Case conceptualisation and treatment plan | 30% | Integrative formulation and planning |
| Observed skills practice with rationale | 20% | Theory-informed intervention and reflection |
| Professional learning portfolio | 15% | Development, ethics, culture, and self-assessment |

A final examination may replace or supplement one component where regulations require. If used, examination questions should require application to unseen cases, not only recall.

## 3. Assignment One: Theory Comparison Essay

**Task.** Compare two theories in relation to adolescent anxiety, depression, school refusal, family conflict, trauma, or another approved presentation. Address view of the person, causation, change, counsellor role, techniques, development, culture, evidence, limitations, and suitability.

**What excellence looks like.** The essay goes beyond parallel summaries. It identifies genuine points of commensurability and conflict, uses evidence accurately, considers a concrete adolescent example, and reaches a justified conclusion.

**Common weaknesses.** Biography replacing analysis; listing techniques without mechanisms; treating evidence as vote counting; assuming adult findings transfer unchanged to adolescents; ignoring family, school, and culture.

## 4. Assignment Two: Evidence Appraisal Brief

**Task.** Select a research article, systematic review, or clinical guideline relevant to one theory or intervention. Appraise question, design, sample, measures, analysis, effect size, limitations, applicability, and implications for youth practice in no more than the assigned length.

Students should distinguish statistical significance from clinical importance and efficacy under trial conditions from effectiveness in ordinary services. They should identify who was excluded from the study and what the intervention required in staffing, training, and time.

A short recommendation section must state: use, adapt, use with caution, or do not use—plus what outcome would be monitored locally.

## 5. Assignment Three: Case Conceptualisation and Treatment Plan

**Task.** Using a provided complex case, produce a developmental, cultural, relational, and evidence-informed formulation and a sequenced treatment plan.

The submission includes:

1. presenting summary and risk;
2. five-part formulation;
3. theory-informed explanation using at least three lenses without incoherent mixing;
4. goals negotiated from client, family, and service perspectives;
5. sequenced interventions with rationale and responsibilities;
6. school, family, health, and safeguarding coordination;
7. outcome measures and review criteria;
8. ethical and cultural analysis;
9. criteria for referral, intensification, or ending.

Word limits reward prioritisation. Appendices may include diagrams and measures but cannot be used to evade core analysis.

## 6. Further Reading for This Chapter


- Biggs, J., & Tang, C. (2011). *Teaching for Quality Learning at University* (4th ed.).
- Race, P. (2020). *Making Learning Happen* (4th ed.).
- Rust, C. (2002). The impact of assessment on student learning. *Active Learning in Higher Education, 3*(2), 145–158.
- Sadler, D. R. (1989). Formative assessment and the design of instructional systems. *Instructional Science, 18*, 119–144.
- Hattie, J., & Timperley, H. (2007). The power of feedback. *Review of Educational Research, 77*(1), 81–112.

## 7. Assignment Four: Observed Skills Practice

**Task.** In a structured practice with a fictional adolescent or family scenario, demonstrate a theory-informed intervention segment and submit a written rationale and reflection.

Assessment focuses on:

- contracting, consent, and safety;
- relationship and listening;
- accurate use of theory-linked skills;
- developmental and cultural responsiveness;
- pacing, boundaries, and response to emotion;
- avoiding harm, coercion, and inappropriate disclosure;
- recognising what did not work;
- identifying next steps.

Students are not assessed on producing a miraculous outcome in fifteen minutes. They are assessed on judgement, process, ethics, and learning. Alternative formats should be available where disability, language, or other access needs require, while preserving the capability being assessed.

## 8. Assignment Five: Professional Learning Portfolio

The portfolio may include:

- a revised formulation showing how feedback changed thinking;
- a cultural humility reflection linked to concrete practice decisions;
- an evidence summary used in teaching or placement;
- a supervision or peer feedback response;
- an ethics analysis;
- a personal development plan with limits of competence.

Reflection should not be graded by amount of personal disclosure. Students may analyse fictional or anonymised material. The criterion is quality of reasoning, connection to evidence and professional standards, and specificity of future action.

## 9. Rubric for Case Conceptualisation

| Criterion | Excellent | Proficient | Developing | Insufficient |
|---|---|---|---|---|
| Formulation | Integrated, developmental, cultural, concise, and testable | Coherent with most relevant factors | Descriptive or partly list-like | Retells case or relies on diagnosis alone |
| Theory use | Theories are compared, sequenced, and linked to mechanisms | Theories are applied accurately | Theory named but weakly linked | Techniques are listed without rationale |

| Criterion | Excellent | Proficient | Developing | Insufficient |
|---|---|---|---|---|
| Planning | Prioritised, feasible, collaborative, and safety-aware | Clear goals and interventions | Generic or overloaded plan | Unsafe, impractical, or missing plan |
| Evidence | Current, critically appraised, youth-relevant | Relevant evidence used accurately | Evidence mentioned superficially | Unsupported claims or adult evidence assumed transferable |
| Ethics and culture | Power, consent, equity, and context shape decisions | Main ethical and cultural issues addressed | Generic statements | Issues ignored or stereotyped |
| Evaluation | Measures, review points, and change criteria are explicit | Outcomes identified | Vague intention to review | No evaluation |
| Communication | Clear, respectful, professional, well organised | Generally clear | Difficult to follow in places | Incoherent or disrespectful language |

Rubrics guide judgement; they do not replace it. Borderline and failing work should receive marker comments tied to criteria and, where institutional rules require, second marking or moderation.

## 10. Rubric for Observed Practice

| Criterion | Indicators of strong performance |
|---|---|
| Contracting | Purpose, confidentiality limits, consent, time, and choices are clear |
| Relationship | Warmth, attention, pacing, and respect are sustained without over-familiarity |
| Theory application | Interventions follow from a stated model and are adapted in real time |
| Developmental fit | Language, autonomy, family context, and capacity are appropriately addressed |
| Cultural responsiveness | Assumptions are checked; identity and power are handled respectfully |
| Safety | Emotion, risk cues, boundaries, and stopping points are managed responsibly |
| Reflection | Student identifies strengths, errors, alternatives, and supervision questions accurately |

A serious ethical breach—such as coercing disclosure, inventing confidentiality promises, or ignoring expressed withdrawal of assent—cannot be offset by fluent technique and should be handled under institutional fitness-to-practise procedures where applicable.

## 11. Feedback, Moderation, and Integrity

Feedback should identify what met the standard, what did not, why it matters, and what to do next. It should arrive while students can still use it. Audio or annotated feedback may help, provided accessibility and records are managed.

Moderation includes calibration meetings, sample second marking, review of grade distributions, external examining where applicable, and analysis of differential outcomes. Markers should discuss anonymised

examples before assessing high-stakes work.

Integrity rules must address plagiarism, contract cheating, unauthorised collaboration, fabricated practice evidence, misuse of client data, and generative AI. Policies should distinguish permitted study support from submitting generated work as one's own analysis. Students should declare assistance where required and remain accountable for accuracy, confidentiality, and reasoning.

No identifiable client material may be placed into external AI systems or public platforms. Even anonymised cases can be identifiable in small communities; institutional guidance governs.

## 12. Accessibility and Fairness

Assessment design should provide clear instructions, examples of structure without model answers that invite copying, reading lists in accessible formats, and reasonable adjustments. Extra time, assistive technology, alternative oral or written formats, and adjusted practice settings should preserve the assessed capability while removing irrelevant barriers.

Group work requires individual accountability and processes for conflict, unequal labour, caring responsibilities, and accessibility. Peer assessment should be formative unless carefully moderated.

Students should have access to criteria before submission, formative opportunities, academic skills support, and transparent late submission, extension, resubmission, and appeals procedures.

## 13. Using Assessment Data

Course teams should review:

- outcome-level performance patterns;
- marker consistency;
- differential attainment and access;
- student feedback and workload;
- skills that remain weak across cohorts;
- whether cases and examples reflect diverse youth contexts;
- whether graduates can transfer theory into practice.

Changes should be documented in course review. If students perform well on essays but poorly in formulation, teaching needs more worked examples and formative case practice. If skills are technically fluent but ethically weak, laboratory supervision and criteria require attention.

## 14. Key Terms

**Key Terms at a Glance**

- **Assessment blueprint** — A map linking outcomes, tasks, weighting, criteria, and quality assurance.
- **Validity** — The extent to which assessment supports intended interpretations and decisions.
- **Reliability** — Consistency of assessment judgements.
- **Moderation** — Processes that calibrate and verify marking standards.
- **Constructive alignment** — Coherence among outcomes, learning activities, and assessment.
- **Fitness to practise** — Professional suitability requirements concerning safety, conduct, and competence.

## 15. Chapter Summary

The assessment system for CAC. 701 should model the counselling it teaches: clear contracting, developmental sensitivity, cultural fairness, evidence-informed judgement, useful feedback, ethical boundaries, and review. Students pass not by sounding like theorists, but by demonstrating that they can use theory to help a young person safely, coherently, and accountably.

## 16. Review and Discussion Questions

**Review and Discussion Questions**

1. Why does assessment reveal a course's real values?
2. Evaluate the proposed weighting for CAC. 701. What would you change and why?
3. Distinguish validity, reliability, fairness, and feasibility.
4. Why should observed practice not be graded mainly on client outcome?
5. What portfolio evidence shows learning without requiring personal disclosure?
6. How should generative AI be addressed in assessment policy?
7. Use rubric data to propose one course improvement.

## 17. Practice Activities

**Practice and Application**

### Activity 58.1 — Blueprint audit

Map an existing course assessment to outcomes and identify unassessed or over-assessed capabilities.

### Activity 58.2 — Marker calibration

Write feedback for a borderline case formulation using the rubric, then compare priorities with a peer.

### Activity 58.3 — Feedback rewrite

Rewrite “Good effort, needs more theory” into feedback identifying standard, evidence, impact, and next action.

## 18. Annotated Reading Pathways

**For assessment design.** Biggs and Tang provide constructive alignment; Sadler and Hattie explain feedback that changes learning.

**For rubrics.** Use rubrics with exemplars, calibration, and narrative comments. A grid without marker judgement creates false precision.

**For integrity and accessibility.** Follow current institutional policy and professional standards, especially where practice material, disability adjustments, and digital tools are involved.

## Practice Laboratory and Extended Worked Example

This laboratory turns **Assessment Blueprint, Assignments, and Rubrics** into decisions that can be observed, supervised, and reviewed. The chapter's central concern is course-level assessment design, major assignments, grading rubrics, and the assurance of academic integrity. In adolescent work, understanding the model is only the first step. The practitioner must also decide what to assess, what to postpone, which adults or institutions need to act, how consent will be maintained, and what evidence would show that the work is helping rather than merely continuing. The organising focus for this part of the course is integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The mechanism to keep in view is how assessment becomes a sequenced, reviewable plan whose parts are matched to mechanisms, people, settings, and evidence. The characteristic professional risk is collecting techniques without coherence, exceeding competence, or allowing documentation and institutional targets to replace the young person's goals. Keep those three sentences available during case discussion. They prevent a common form of drift in which a practitioner uses familiar language from a theory while the actual session is being governed by urgency, institutional pressure, or the wish to make distress stop quickly.

### A twelve-part application map

The following map is not a rigid manual. It is a way of checking whether the work has a beginning, a middle, a reviewable middle-late phase, and an ending. Some young people need only selected elements; others require a much longer stabilisation period or referral before the central method is appropriate.

| Phase | Practitioner task | Young person's experience | Evidence to review |
|---|---|---|---|
| 1. Contract | Explain purpose, confidentiality limits, choices, session rhythm, and complaint or advocacy routes. | "I know what this is and what may be shared." | Consent/assent record and the young person's own summary. |
| 2. Map contexts | Gather home, school, peer, digital, health, cultural, and material contexts relevant to the referral. | "Adults are not treating one setting as my whole life." | A context map with missing information identified. |
| 3. Identify patterns | Describe sequences, triggers, responses, consequences, strengths, and exceptions in observable language. | "The problem has a pattern; I am not simply the problem." | Baseline notes, diaries, measures, or functional records. |
| 4. Share formulation | Offer a tentative explanation in plain language and invite correction from the young person and, where appropriate, family. | "This account fits, partly fits, or needs changing." | A dated formulation with disagreements recorded. |
| 5. Agree targets | Select a small number of meaningful, feasible, and reviewable goals. | "These goals are mine as well as the service's." | Goal ratings and functional outcomes. |
| 6. Build capacity | Teach, rehearse, model, or arrange the relational and environmental supports required before demanding change. | "I have a way to begin, not just an instruction." | Observed practice, mastery, and confidence ratings. |
| 7. Apply the method | Use the chapter's core procedures in case conferences, supervision, school and clinic teams, teaching sessions, and service review, adapting dose and pace to development and state. | "The work is challenging but understandable and stoppable." | Session records, fidelity notes, and client feedback. |
| 8. Transfer to real life | Plan practice in the settings where change must survive, with adults and peers given clear, limited roles. | "This is not only something I do in a counselling room." | Home, school, or community evidence of generalisation. |
| 9. Review obstacles | Treat non-completion, rupture, or deterioration as information about fit, sequencing, context, or risk. | "A setback leads to inquiry, not blame." | Revised formulation and documented changes. |
| 10. Involve systems | Use consultation, advocacy, family work, or care coordination where the maintaining conditions lie outside the individual session. | "Adults are changing some conditions too." | Meeting actions, school adjustments, and referral outcomes. |
| 11. Consolidate | Name the strategies, relationships, and environmental changes that produced improvement. | "I can explain what helps and why." | Relapse or setback plan in the young person's language. |
| 12. End or transfer | Review gains, unfinished needs, future help routes, and information transfer with consent. | "Ending is planned; I am not disappearing from care." | Closing summary, measures, and follow-up arrangements. |

## Session-by-session guide

**Session one: make the work intelligible.** Begin with the young person's account before offering the theory. Ask what they have been told about the referral, what they expect, what would make sessions unbearable,

and what a useful change would look like in an ordinary week. Explain confidentiality in concrete categories: what remains in counselling, what may be summarised for agreed support, and what may need to be shared for safety. End by checking the explanation back in the young person's words. A technically correct contract that the young person cannot repeat has not yet been understood. **Session two: widen the lens without losing focus.** Map the problem across settings. A difficulty that appears only under public evaluation differs from one that follows the young person everywhere. Record sleep, health, learning, language, family routines, peer contact, digital pressures, and previous help. Ask what adults currently do that helps even slightly, and what they do with good intentions that accidentally maintains the pattern. This session should produce questions, not premature certainty. **Session three: build a shared formulation.** Offer no more than three linked mechanisms at first. For example: a trigger, an interpretation or bodily response, a protective action, and a consequence that brings short-term relief but longer-term cost. Draw the sequence together. Ask the young person to rename any professional term that feels wrong. If parents or school staff will receive a version, agree its content and purpose. The formulation is a working map, not a verdict. **Session four: choose the first experiment.** Select a step small enough to be attempted before the next meeting and important enough to matter. Define who will do what, where, for how long, and how the result will be noticed. Rehearse in session if the step is interpersonal. Predict obstacles without treating them as excuses. A young person who cannot attempt the step has provided information about grading, support, timing, or goal ownership. **Sessions five and six: practise, review, and adjust.** Review actual attempts before introducing new material. What happened first, what followed, and what was learned? Strengthen successful components and revise those that depended on unrealistic adult consistency. If family or school responses undermined the plan, do not solve the problem by asking the young person to try harder inside unchanged conditions. Use consultation or a joint meeting with a narrow purpose. **Sessions seven and eight: deepen without flooding.** By this stage the practitioner may work more directly with emotion, memory, belief, family sequence, behaviour, identity, or relationships, depending on the model. Deepening does not mean intensity for its own sake. Watch arousal, dissociation, shame, and post-session functioning. A session that produces dramatic disclosure followed by school absence, family conflict, or increased self-harm requires reformulation and possibly a slower phase. **Sessions nine and ten: generalise and distribute support.** Move the work into ordinary life. Identify which teacher, parent, carer, mentor, coach, peer, or community adult can support the new pattern without becoming a therapist. Keep instructions brief and observable. The young person should know exactly what information those adults hold. Generalisation fails when every adult improvises a different version of the plan. **Final sessions: consolidate authorship and plan for setbacks.** Review the original goals and the evidence of change. Ask the young person to explain, in their own words, what the problem does, what helps, what early signs matter, and what they will do first if it returns.Practise a likely setback rather than treating relapse as failure. Close with a summary that the young person has seen, a clear route back to help, and transfer of relevant supports to family, school, or community systems.

## Worked example for seminar discussion

Tobias is fifteen and is seen in an integrated school and health team for difficulty trusting adults after several service changes. The referral describes the problem in institutional language and recommends that counselling should improve motivation. Tobias's own first statement is different: “Adults keep asking what is wrong with me, but nobody asks what happens just before it goes wrong.” That sentence becomes the assessment task. The practitioner first establishes safety, consent, and context. Tobias explains that the difficulty is worst when tired, observed by peers, and uncertain about what an adult expects. At home,

concern is expressed through repeated questioning; at school, public correction increases urgency. A previous professional focused only on the visible behaviour and recommended consequences. The new formulation instead asks how state, interpretation, relationships, and consequences interact, consistent with this chapter's focus on integration, formulation, planning, measurement, coordination, ethics, and professional judgement. The first intervention is deliberately modest. Tobias chooses one setting in which to test a new response, one adult who will respond predictably, and one way of recording what happens. The practitioner rehearses the step and agrees what will count as useful information even if the attempt fails. Parents are not given session content; they are given a narrow role: reduce repeated prompting, notice the agreed attempt, and report practical obstacles at a planned review. School receives only the functional adjustment Tobias has approved. At review, the result is mixed. The chosen step works twice and fails once after a night of poor sleep and a peer comment. A weak practitioner might call this inconsistency. The formulation instead becomes more precise: capacity is state-dependent, peer audience changes the task, and adult responses need to be steadier. The next step is redesigned rather than enlarged. This is the clinical discipline the laboratory is intended to teach: evidence from the young person's life revises the plan. By the later sessions, Tobias can describe the pattern without adopting the referral label. Improvement is measured not only by symptom frequency but by participation, recovery time, willingness to seek help, and the adults' ability to respond without escalation. The ending plan names early signs, first responses, trusted people, and the point at which professional help should be sought again. The case remains fictional and simplified, but it demonstrates the movement from explanation to negotiated action.

## Family, school, and cultural translation

Translate the theory into the languages of the systems around the young person. Parents need to know what to do differently on Tuesday evening, not only what the model is called. Teachers need observable adjustments: timing, audience, instruction format, feedback privacy, break arrangements, or response to early signs. Community and health professionals need the specific decision requested from them. A plan that requires everyone to understand the whole theory will fail; a plan that assigns each person one coherent action may succeed. Cultural translation is not achieved by inserting a stereotyped example. Ask how the young person and family explain the difficulty, who is trusted, what privacy means, which adult decisions are collective, what language carries the relevant concepts, and what previous experiences with services shape caution. Check whether the intervention asks the least powerful person to absorb all change. If the school timetable, housing, transport, discriminatory response, or family workload is maintaining distress, consultation and advocacy belong in the treatment plan. Developmental translation requires attention to language, abstract reasoning, attention, memory, autonomy, and dependence. A method designed for adults may need shorter tasks, visual materials, movement, caregiver involvement, school coordination, and more explicit consent. Adaptation is not dilution when the mechanism is preserved and made usable; it is dilution when only the vocabulary is borrowed.

## Common errors and supervision questions

- Using the theory's label as an explanation: “She does that because she is resistant, avoidant, dysregulated, or unmotivated.” Ask what was observed and what else could explain it.
- Selecting an intervention because it is familiar to the practitioner rather than matched to mechanism, evidence, development, and consent.

- Involving family or school so broadly that the young person's confidential space disappears, or excluding them so completely that the environment defeats the work.
- Interpreting lack of progress as client failure before checking fidelity, dose, practical barriers, alliance, risk, and formulation accuracy.
- Moving faster because the service has few sessions, without making the compression explicit or arranging step-down support.
- Recording speculation as fact, especially about motives, trauma history, identity, family intent, or risk.
- What exactly did the young person say they wanted, and how does the plan preserve that aim?
- Which mechanism is the current intervention supposed to change, and what would count as evidence?
- What has been ruled out medically, educationally, culturally, and in safeguarding terms?
- Who else must change a response for the young person's new behaviour to survive?
- What is the practitioner feeling pulled to do—rescue, punish, withdraw, persuade, avoid—and how is that being supervised?
- If only three sessions remained, what should be consolidated rather than newly opened?
- What information does the young person believe is private, and does the record match that understanding?

## Outcome check

Review at least four kinds of outcome. First, **experience**: does the young person feel safer, understood, and more able to choose? Second, **function**: what is happening in learning, attendance, family life, friendship, sleep, and participation? Third, **mechanism**: has avoidance reduced, regulation improved, communication changed, mastery increased, family accommodation shifted, or support become more reliable? Fourth, **system**: are adults and institutions responding differently, and will those changes survive staff absence or term transitions? A symptom score may contribute to each judgement; it cannot replace them. End the laboratory by writing a five-sentence plan: the young person's goal; the formulation in ordinary language; the next intervention and who will carry it; the main ethical or cultural caution; and the date and evidence for review. If those five sentences cannot be written clearly, the theory has not yet become practice.

## Advanced seminar exercises for Assessment Blueprint, Assignments, and Rubrics

**Compare two explanations.** Write two competing explanations of the same adolescent difficulty using **Assessment Blueprint, Assignments, and Rubrics** and one other approach from this book. Each explanation must identify a mechanism, an intervention, a predicted early sign of improvement, and an observation that would weaken the explanation. The exercise prevents theoretical loyalty from replacing reasoning: a model earns its place by the decisions it changes.

**Audit the first three sessions.** Design the first three contacts for a fictional adolescent. State what will be explained, what will not yet be attempted, which adults may be involved, and what record will be created. Pay particular attention to assent, privacy, developmental language, and the difference between the referrer's goal and the young person's goal. Many treatment failures are contracted in the first meeting long before a recognisable technique is used.

**Find the maintaining loop.** Draw a loop with at least five points: context, bodily state or appraisal, protective action, other people's response, and longer-term consequence. Mark where **Assessment Blueprint, Assignments, and Rubrics** would intervene and where a family, school, health, cultural, or practical intervention is more appropriate. Require every arrow to be supported by an observation or labelled as a hypothesis.

**Practise the difficult conversation.** In triads, rehearse explaining the approach to a sceptical adolescent, a worried parent, and a teacher who wants a quick behavioural result. The practitioner must use ordinary language, state limits, invite correction, and avoid promising outcomes. Observers record where jargon, coercion, or false certainty enters. Repeat the conversation after feedback; fluency under challenge is a professional skill.

**Examine power and access.** Ask who defined the problem, who attends, who can refuse, whose record is believed, who pays or travels, whose language is used, and who carries the burden of change. Consider disability, poverty, race, gender, sexuality, care experience, migration, and school status where relevant to the fictional case. Identify one institutional action that should accompany any individual intervention.

**Plan the review.** Choose four outcomes: the young person's experience, daily functioning, the mechanism targeted by **Assessment Blueprint, Assignments, and Rubrics**, and the response of the surrounding system. Specify when each will be reviewed and what decision a disappointing result would trigger—reformulate, adjust dose, involve others, consult, refer, pause, or end. This turns evaluation from a discharge ritual into clinical governance.

**Written consolidation.** In 250 words, explain Assessment Blueprint, Assignments, and Rubrics to a fourteen-year-old, a parent, and a teacher without using the theory's technical label. The three versions should preserve the same core idea while changing vocabulary, examples, and responsibilities. If the explanation only works when the listener accepts the professional's authority, it is not yet clinically usable.

CHAPTER APPENDIX A

# Consent, Assent, and Records Forms

Model documentation for minors, families, and schools.

## A1. Purpose and Use of These Forms

These model forms support counselling with children and adolescents in educational, community, clinic, and private-practice settings. They are teaching templates, not legal advice. Before use, each service must adapt them to current local law, professional regulation, safeguarding policy, record-retention rules, insurance requirements, and organisational governance. Where a young person's safety or legal rights are involved, qualified local advice should be obtained.

Consent is not a signature obtained once and forgotten. It is an ongoing process of explanation, understanding, choice, review, and withdrawal. For minors, legal consent may rest with a parent, guardian, court, or the young person, depending on age, capacity, jurisdiction, and the nature of the service. Assent is the young person's affirmative agreement to participate even where another person provides legal consent. Dissent—refusal, withdrawal, distress, silence, or persistent non-attendance—must be noticed and explored rather than overridden merely because paperwork has been signed.

A form should never promise what the service cannot deliver. Do not promise absolute confidentiality, immediate crisis response outside operating hours, permanent record destruction contrary to retention law, or access to a particular practitioner if staffing cannot guarantee it.

## A2. Core Elements of a Consent Record

A complete counselling consent record should identify:

- the legal name of the young person and the name they wish to be used in sessions and communications;
- date of birth or age range where required for service eligibility;
- parent, guardian, or legal decision-maker details where applicable;
- the service name, practitioner name and role, supervisor or accountable lead where relevant;
- purpose and likely nature of counselling;
- session length, frequency, location or remote platform;
- fees, cancellation rules, and funding source where applicable;
- benefits, alternatives, and foreseeable limitations;
- confidentiality and its limits;
- record creation, access, storage, retention, and sharing;
- communication methods and response times;
- safeguarding and emergency arrangements;
- complaint and advocacy routes;
- the right to ask questions, pause, withdraw, or seek another service;
- signatures, dates, interpreter details, and any adjustments provided.

Where consent is obtained verbally because of literacy, disability, remote access, or urgent circumstances, record who explained the information, what was explained, how understanding was checked, who agreed, and any witnesses or interpreters used under policy.

## A3. Model Information Statement for Parents and Carers

**Service name:** _____

**Practitioner:** _____

**Young person:** _____

Counselling offers a planned space in which a young person can discuss concerns, understand patterns, develop coping and communication skills, and work toward agreed goals. The counsellor may meet the young person alone, meet parents or carers separately, and, where agreed and clinically appropriate, hold joint conversations. The counsellor does not act as a disciplinary officer, custody evaluator, academic tutor, or investigator unless a separate role has been explicitly commissioned and explained.

Sessions normally last _____ minutes and occur _____. The initial plan will be reviewed on or before _____. Progress is evaluated using conversation, agreed goals, functioning at home and school, and, where appropriate, brief questionnaires.

The counsellor will keep records necessary for safe and accountable care. Records are stored by _____ and retained according to _____. Parents and carers may have rights to information under local law and policy, balanced against the young person's confidentiality, safety, and developmental capacity. The counsellor will discuss information-sharing arrangements before counselling begins.

Confidentiality will be maintained except where disclosure is required or permitted to protect the young person or another person from serious harm, to comply with safeguarding procedures or law, or where the young person agrees to sharing. Where possible, the counsellor will discuss proposed sharing with the young person first.

Parents and carers are asked not to require the young person to report session content after appointments. Helpful parental involvement includes supporting attendance, communicating practical concerns, participating in agreed parent sessions, and informing the service promptly about major risks or changes.

In an emergency outside session hours, contact _____. The counselling service does not provide continuous crisis monitoring unless explicitly stated here: _____.

I confirm that the service, its limits, record arrangements, fees or funding conditions, complaint route, and confidentiality limits have been explained in a language and format I could use. I have had an opportunity to ask questions.

Name: _____ Relationship/legal authority: _____

Signature: _____ Date: _____

Interpreter or communication support used, if any: _____

## A4. Young Person's Assent Statement

My counsellor is _____. I understand that counselling is a time to talk, think, practise skills, or use other agreed ways of working on things that matter to me.

I understand that:

- I can say what I think the problem is and what I want to change.

- I can ask what will happen with information I share.
- What I say is private unless the counsellor needs to share information to keep me or someone else safe, or the law/policy says information must be shared.
- The counsellor may need to speak to adults about practical arrangements or safety, but we will discuss what will be said whenever possible.
- I can ask for a pause, say I do not want to answer, or ask for a different way of working.
- I can ask who keeps notes, where they are kept, and how to complain or get support from another adult or advocate.
- If sessions are online or by phone, we will agree where I will be, who can hear, what happens if the connection fails, and how I can contact help in an emergency.
- I can change my mind about counselling. If I stop coming, the counsellor or service may contact me or my parent/carer in the way we agreed, especially if they are worried about safety.

My preferred name in sessions is: _____

Adults may use this name in these settings: _____

Adults should not use this name or share information about counselling in these settings without discussing it with me first: _____

The best safe way to contact me is: _____

I agree to take part in counselling at this time.

Young person's signature or mark: _____ Date: _____

Where a young person does not wish to sign, record assent discussion, questions, preferences, and any dissent or conditions below.

Practitioner record: _____

## A5. Capacity and Communication Checklist

Before relying on consent or assent, consider and record:

- Can the young person understand the purpose and nature of counselling in accessible language?
- Can they retain the main choices long enough to decide?
- Can they weigh obvious benefits, burdens, and alternatives?
- Can they communicate a preference by speech, sign, writing, device, behaviour, or trusted support?
- Are pain, medication, fatigue, intoxication, distress, language, disability, or another person’s presence affecting apparent agreement?
- Has an interpreter, advocate, or communication aid been offered where needed?
- Has the young person had an opportunity to speak without a parent, teacher, partner, or peer present?
- Are there signs of coercion by family, school, partner, peers, or professionals?
- What adjustments were made, and what remains uncertain?

Capacity is decision-specific and may fluctuate. A young person may be able to choose session activities but need support for record-sharing decisions, medical referrals, or complex family meetings.

## A6. Review and Withdrawal Record

Consent and assent should be reviewed when:

- goals or methods materially change;
- a new person joins treatment;
- family or school information-sharing changes;
- teletherapy begins or ends;
- recording, testing, or digital tools are introduced;
- risk or safeguarding status changes;
- the young person reaches a relevant legal age or changes placement;
- treatment extends beyond the period originally explained;
- the young person expresses doubt, dissent, or misunderstanding.

Date of review: _____ People present: _____

What changed: _____

What the young person said: _____

What was agreed: _____

Withdrawal or pause requested by: _____

Safety or continuity actions taken: _____

Referral or information provided: _____

Practitioner signature: _____ Date: _____

## A7. Common Errors to Avoid

Do not use a parent's signature to silence an adolescent's dissent. Do not obtain consent for “counselling” and then introduce trauma processing, group participation, recording, testing, or school reporting without renewed explanation. Do not state that records are confidential if school administrators, insurers, digital platforms, or courts may access them under policy or law. Do not guarantee that a young person can withdraw from safeguarding action. Do not make counselling a condition of avoiding punishment without explaining the mandated frame and preserving meaningful choices inside it.

The ethical test is simple: if the young person, parent, or future reader of the record saw exactly what had been explained and agreed, would they recognise the service that followed?

## A2.1 Confidentiality Agreement for Counselling with Minors

This model should be adapted to law and service policy. It is designed to be read aloud and discussed, not merely signed.

## What stays within counselling

Session conversation, questionnaires, artwork, digital material created for therapy, and the counsellor's process reflections are treated as confidential clinical information. Access is limited to authorised professionals with a legitimate role under policy.

## What may be shared routinely with agreement

Practical information may include attendance, appointment changes, broad progress, agreed goals, safety plans, and recommendations for home or school. The young person should know the wording or substance whenever possible.

Routine sharing agreed:

With parent/carer: _____

With school: _____

With health or social care professionals: _____

Other: _____

## What may or must be shared without agreement

Information may be shared where there is serious risk of harm, suspected abuse or exploitation, a legal duty, a court order, or another lawful safeguarding basis. Only relevant information should be shared, with the minimum necessary audience. Unless doing so would increase danger or obstruct protection, the young person will be told what is being shared, with whom, and why.

## Records

Records may include referral information, consent, assessments, formulations, session summaries, risk and safety plans, correspondence, measures, and endings. They will be stored at _____, retained for _____, and accessed under _____.

The young person and/or parent may request access, correction of factual inaccuracies, or an explanation of record rights through _____. Some third-party or safeguarding information may be limited by law or policy.

## Digital communication

Agreed channels: _____

Response hours: _____

Channels not to be used for clinical or crisis communication: _____

If technology fails during a remote session, the counsellor will: _____

## Signatures and discussion record

Young person: _____ Date: _____

Parent/carer or legal representative, where required: _____

Practitioner: _____ Date: _____

Discussion notes, disagreements, and unresolved questions: _____

## A2.2 Information-Sharing Decision Record

Use one record for each significant non-routine disclosure.

Date and time: _____ Decision-maker: _____

Information proposed for sharing: _____

Source and reliability of information: _____

Purpose of sharing: _____

Legal/policy basis: _____

Young person's views: _____

Parent/carer views, if relevant and safe: _____

Risks of sharing: _____

Risks of not sharing: _____

Minimum information and recipients: _____

Was the young person informed before sharing? Yes / No

If no, why: _____

What was actually shared, by whom, and how: _____

Follow-up required: _____

Supervisor or safeguarding lead consulted: _____

## A2.3 Session Record Template

Client identifier: _____ Date: _____ Session no: ___

Present and setting: _____

Purpose/agenda agreed: _____

Young person's account in their own key words: _____

Observations relevant to care (separate from interpretation): _____

Assessment/formulation update: _____

Interventions used and response: _____

Risk, safeguarding, health, or school issues: _____

Parent/school/professional contact, with consent or lawful basis: _____

Agreements, homework, or practice: _____

Outcome measure or goal rating, if used: _____

Plan and next appointment: _____

Information shared after session: _____

Practitioner signature: _____ Date/time of entry: _____

Records should be written promptly, accurately, respectfully, and without unnecessary intimate detail. Distinguish what was reported, observed, inferred, and decided. Avoid labels such as manipulative, attention-seeking, resistant, or non-compliant without behavioural evidence and context.

## A2.4 Assessment Summary Template

Reason for assessment and decisions it must inform: _____

Sources used and missing sources: _____

Young person's views and goals: _____

Development, health, learning, language, and access needs: _____

Family, peer, school, digital, cultural, and community context: _____

Presenting pattern by setting: _____

Strengths and previous helpful responses: _____

Risk and protective formulation: _____

Standardised measures, scores, limitations, and interpretation: _____

Formulation: _____

Recommendations and responsible persons: _____

Information-sharing plan: _____

Review date and criteria: _____

## A2.5 Referral Letter Template

Dear _____,

I am referring _____ (date of birth/age and preferred name where relevant and consented) for assessment/support with _____.

Reason for referral and decision requested: _____

Relevant background: _____

Current presentation and functioning: _____

Risk and safeguarding information relevant to your role: _____

Strengths, preferences, communication and access needs: _____

Work already undertaken and response: _____

Current professionals and family arrangements: _____

Specific questions for your service: _____

Consent/assent and information-sharing basis: _____

Attachments: _____

Please contact _____ by _____ if further information is required.

Name, role, service, secure contact: _____

## A2.6 Closing Summary Template

Episode dates and number of sessions: _____

Reason for counselling and initial goals: _____

Main formulation in respectful summary form: _____

Interventions and participation: _____

Changes in symptoms, functioning, relationships, participation: _____

Young person's view of progress and remaining concerns: _____

Risk and safety plan at closure: _____

Recommendations for family/school/other services: _____

Referrals made and accepted: _____

How to seek help again: _____

Copies provided to: _____

Practitioner: _____ Date: _____

## A2.7 Record Access and Correction Log

Date request received: _____ Requester and authority: _____

Identity verified by: _____

Records requested: _____

Legal/policy review by: _____

Material released, withheld, or redacted and reasons: _____

Date and method of release: _____

Correction requested: _____

Outcome and notification: _____

Complaints or advocacy information supplied: _____

## A2.8 Retention and Destruction Note

Services must not promise destruction at a date contrary to law, safeguarding requirements, insurance, or organisational retention schedules. When records reach review, record the schedule applied, reviewer, decision, method of secure destruction where authorised, and any legal hold. Digital deletion should include relevant backups and vendor systems as far as contractually and technically possible, with limitations documented.

## A3.1 Teletherapy Consent and Assent Addendum

Counselling may be offered by secure video, telephone, or another approved method. Remote counselling has benefits and limits. Privacy, technology, location, emergency response, and observation may differ from in-person care.

Platform approved by service: _____

Young person's location during sessions: _____

Address or location description to be confirmed at each session: _____

Emergency contact: _____ Local crisis resource: _____

Backup telephone number: _____

If the connection fails, the practitioner will: _____

If the practitioner is concerned about immediate safety, they may contact: _____

People who may be present or nearby: _____

Agreed privacy arrangements and signal if the young person cannot speak freely: _____

Camera expectations and alternatives: _____

Recording, screenshots, automated transcription, and AI tools are not permitted unless separately explained and authorised here: _____

Messaging boundaries and expected response times: _____

Parent/carer role during remote sessions: _____

I understand that I may request in-person or another format where available, and that suitability will be reviewed if privacy, safety, technology, or clinical needs change.

Young person: _____ Date: _____

Parent/carer or legal representative where required: _____

Practitioner: _____ Date: _____

## A3.2 School Counselling Information Statement

School counselling is a supportive service. It is not a disciplinary interview, behaviour sanction, academic assessment, or investigation. The school counsellor may provide brief counselling, consultation to staff, family liaison with appropriate consent, group work, crisis support, and referral.

Students may be referred by staff, family, or themselves. Referral does not mean the student is in trouble. The counsellor will explain who made the referral and why in developmentally appropriate terms.

Session content is confidential within professional and legal limits. Teachers may receive practical recommendations or broad progress information where the student agrees or where policy permits. Teachers will not ordinarily receive transcripts or detailed disclosures. Safeguarding information will be passed to designated staff promptly.

School records created by the counselling service are stored: _____

Staff who can access counselling records: _____

Appointment system and how privacy is protected: _____

Parent/carer notification practice for this age/service: _____

Complaint or advocacy route: _____

The student may ask what information is held, who can see service summaries, and how factual errors can be corrected under policy.

## A3.3 Group Counselling Agreement

Group purpose: _____

Members are expected to attend on time, respect different experiences, avoid recording or screenshots, and not repeat other members' personal disclosures outside the group. The facilitator cannot guarantee that every member will maintain confidentiality; members should share only what they are prepared to have heard in a group.

Members may pass on an activity, ask for a pause, or speak to the facilitator individually. Threats, harassment, sexual or discriminatory comments, and pressure to disclose are not permitted. Safeguarding limits apply.

For online groups, members must join from a private-enough location, use the agreed name display, keep cameras or microphones as agreed, and not invite unapproved people into the session.

Young person's agreement: _____ Date: _____

Parent/carer consent where required: _____ Date: _____

Facilitator: _____ Date: _____

## A3.4 Recording, Observation, and Training Consent

No audio, video, automated transcription, live observation, or use of material for teaching or research should occur without separate, specific consent and assent where required. Consent for treatment does not imply consent for recording or publication.

Purpose of recording/observation: _____

Who will access it: _____

Where and how long it will be stored: _____

Whether excerpts may be used in supervision, training, assessment, or research: _____

Whether the young person/family may stop recording: _____

Date after which consent expires: _____

I understand that declining recording or observation will not reduce access to ordinary care, except where a specific training clinic model has been explained before acceptance into that service.

Signatures: _____

Withdrawals and destruction actions should be logged. Even anonymised case material can be identifiable; teaching use requires disguise, minimisation, and institutional approval.

## A3.5 Consent for Standardised Testing or Digital Tools

Instrument/tool name: _____

Purpose and decision it will inform: _____

What the young person will do and how long it will take: _____

Alternatives: _____

Limitations, language and cultural considerations: _____

Who will see raw data, scores, reports, and vendor-processed data: ____

Data location, retention, deletion, and secondary use: _____

Feedback arrangements: _____

Consent/assent signatures and date: _____

A score must be interpreted by a competent practitioner in context. Automated recommendations, including AI-generated outputs, require human review and must not be the sole basis for diagnosis, safeguarding, school placement, or treatment decisions.

## A3.6 Consent Where Parents Are Separated or Legal Authority Is Disputed

Record, from reliable documentation where available:

- persons with parental responsibility or legal authority;
- court orders affecting contact, health, education, residence, travel, or information;
- who may consent to counselling and receive information;
- safeguarding restrictions on addresses, schools, photographs, or contact;
- who should receive appointment messages;
- any disagreement and legal advice or court direction obtained.

Do not rely solely on one adult's account where documents are available and conflict is known. Do not disclose a protected address or school. Where parents disagree about non-urgent counselling, follow local law and policy, consider the young person's views and best interests, and obtain legal or senior advice where necessary.

The young person should not be used as messenger between adults. Session arrangements should avoid placing them in loyalty conflicts.

## A3.7 Mandated Counselling Statement

You have been referred by _____. Attendance or participation may be required by _____. The counsellor will explain what attendance information, if any, must be reported, to whom, and in what words.

Within that requirement, you may still make choices about goals, pace, topics, and methods where clinically and legally possible. The counsellor will not pretend that mandated attendance is fully voluntary. Concerns

about the mandate can be raised through _____.

The counsellor's clinical opinions and reports, if any, are limited to: ____

Young person's acknowledgement of explanation: _____

This statement is especially important in school discipline, court, child protection, and employer- or insurer-funded contexts.

## A3.8 Periodic Consent Review Prompts

At review, ask:

- Do you still understand why counselling is happening?
- What has been helpful, unhelpful, surprising, or uncomfortable?
- Is there anything you believed would stay private that has been shared?
- Has who you want involved changed?
- Has the safest contact method, name, pronoun, address, school, or platform changed?
- Do you want to continue, pause, change method, involve someone new, or end?
- Are there records or summaries you want explained or corrected?
- What should happen if you stop attending?

Document the answers in the young person's words where material. Consent is alive only if review can alter practice.

CHAPTER APPENDIX B

# Case Study Portfolio

Twelve extended composite case studies for seminar use.

## B1. How to Use This Portfolio

The cases in this appendix are fictional composites created for teaching. Any resemblance to an identifiable person is unintended. They are designed to help students integrate theory, development, culture, ethics, and planning. Names, settings, and details should not be copied into real records.

For each case, students should produce:

1. a concise presenting summary;
2. a five-part formulation;
3. at least two competing theoretical readings;
4. a risk and safeguarding analysis;
5. a sequenced treatment plan;
6. family, school, and service coordination;
7. outcome measures and review points;
8. a young-person-friendly explanation of the plan;
9. an account of what the practitioner might be getting wrong.

The portfolio is not a test of finding one hidden diagnosis. Strong work tolerates uncertainty, identifies missing information, and states what observation would change the formulation.

## Case 1: The Boy Who Arrived After the Bell

**Age and context.** Tunde, thirteen, lives with his mother and younger sister. His father works abroad for long periods. Tunde was a regular attender in primary school. Since moving to secondary school he arrives late three or four days a week, misses registration, and has received repeated detentions. Teachers describe him as bright, funny, and increasingly rude when challenged publicly.

**History.** Tunde changes buses once. The second bus is unreliable. At home, his mother leaves for work before he wakes. His nine-year-old sister depends on him to check that the door is locked and that she has her school bag. Tunde stays up late playing online games with cousins in another time zone. He says mornings feel “impossible” and that detention is “better than walking in when everyone looks”.

**Observations.** He concentrates well in afternoon lessons. He is popular but avoids reading aloud. Screening suggests possible dyslexic difficulties not yet assessed. His mother feels blamed by school letters. She says Tunde “chooses games over his future”. Tunde says adults only contact home when he is in trouble.

**Student tasks.** Separate lateness as behaviour from explanations. Analyse sleep, transport, caring responsibility, public shame, literacy, family communication, and school sanctions. Design a plan that does not make Tunde responsible for adult transport failures. Identify what should change at home, in school systems, and in Tunde's routines. State what information is missing about his father's role, the sister's school arrangements, and the journey.

**Teaching point.** A behaviour can be simultaneously a skill problem, a systems problem, a developmental problem, and a rational response to humiliation. Contingency management without ecological change would be efficient-looking and unjust.

## Case 2: The Girl Who Became the Family Interpreter

**Age and context.** Mei, fifteen, migrated with her parents and grandmother two years ago. She is fluent in the school language and interprets for medical appointments, housing letters, and parent evenings. Teachers praise her maturity. She has developed stomach pain, declining grades, and anger at home.

**History.** Her mother has chronic illness; her father works nights. Mei's grandmother tells stories about the family's former home and worries that Mei is "losing respect". Mei has stopped attending an art club because appointments and homework translation take priority. She says, "If I don't do it, nobody understands anything. If I do it, I get told things a child shouldn't know."

**Risk and needs.** No suicidal thinking is reported. She has missed meals before examinations and says pain improves at weekends. Medical assessment has not yet occurred. Her parents value education highly and feel ashamed that services know about family difficulties.

**Student tasks.** Formulate parentification, migration loss, language brokering, gender expectations, somatic symptoms, and school pressure. Identify professional interpreter needs and medical steps. Design family work that redistributes adult responsibilities without shaming parents. Consider how to ask Mei what she wants adults not to know through her interpreting role.

**Teaching point.** Competence can conceal burden. A strengths-based reading that only celebrates maturity will collude with the system extracting resilience from her.

## Case 3: Two Homes, Two Rules, One Phone

**Age and context.** Leo, fourteen, lives alternately with separated parents. His mother permits phone use until 10 p.m.; his father removes all devices on arrival. Leo has begun keeping a second phone given by an older cousin. School reports conflict after weekends at his father's home.

**History.** The separation followed high conflict. Each parent asks Leo what happens at the other home. Leo tells each what reduces immediate questioning. His grades are stable, but he has stomach aches on transition days and has started refusing to go to his father's home, then crying afterward because he misses him.

**Student tasks.** Map loyalty binds, transitions, surveillance, autonomy, peer contact, and inconsistent contingencies. Distinguish safety monitoring from interrogation. Draft a parent-only meeting agenda that does not use Leo as messenger. Identify what information the counsellor should and should not carry between households.

**Teaching point.** Consistency does not require identical households. Children can manage different rules when transitions are predictable, adults communicate directly, and differences are not framed as proof that one home is corrupt.

## Case 4: The Student Who Was "Fine" All Day

**Age and context.** Amelie, twelve, is quiet, compliant, and academically successful. At home she cries for long periods, refuses food on some evenings, and repeatedly asks whether her parents are angry. Teachers are surprised by the referral.

**History.** Her parents separated amicably but both work long hours. Amelie cares for a younger brother after school. She worries about saying the wrong thing and checks doors and messages repeatedly. She says

school is easier because “the rules are on the wall; at home I have to guess”.

**Student tasks.** Explore masking, anxiety, reassurance, perfectionism, caring responsibilities, family emotional communication, eating and medical assessment, and the risk of schools dismissing distress because attainment is high. Plan parent work and graded reduction of reassurance without abrupt withdrawal of comfort.

**Teaching point.** Functioning is setting-specific. High achievement can coexist with severe distress and may delay recognition.

## Case 5: The Assembly

**Presentation.** Sena, thirteen, vomited before a school assembly after being asked to read. She now avoids corridors, group work, presentations, and the dining hall. She can speak comfortably with two friends and at home.

**Data.** Her attendance is 62 per cent. She fears vomiting and being watched swallowing. Parents initially allowed her to remain home on presentation days. A teacher once commented, “We haven't got all day,” when she paused. She has no eating disorder symptoms reported, but medical causes for nausea have not been assessed.

**Tasks.** Conduct differential assessment among social anxiety, panic, specific phobia, trauma response, medical problems, and school-based humiliation. Build a hierarchy that starts with success and includes checking feared outcomes. Analyse parental accommodation and teacher practices. Identify measures beyond attendance.

**Expected integration.** CBT/exposure principles, social cognitive mastery, IPT-A peer issues, school consultation, family work, and developmental sensitivity to shame.

## Case 6: The Empty Calendar

**Presentation.** Jonas, sixteen, stopped attending college after failing two assessments. He sleeps until afternoon, games at night, and says he is “not depressed, just done”. His mother finds unopened letters from college. He has lost contact with friends.

**Data.** He previously wanted to study engineering. He describes every task as evidence that he is behind. He avoids messages because they contain deadlines. He denies suicidal intent but says the future feels “blank”. Appetite and concentration are poor.

**Tasks.** Assess depression, suicide risk, substance use, sleep, learning problems, physical health, and family responses. Construct a behavioural activation plan with mastery and social contact. Use motivational interviewing to explore the apprenticeship/college discrepancy. Identify how academic backlog should be triaged so return does not recreate failure.

**Teaching point.** Avoidance may protect against shame in the short term while removing every source of reward and competence.

## Case 7: The Perfect Score

**Presentation.** Hana, fifteen, studies five hours nightly, cries over minor errors, checks homework repeatedly, and refuses to submit work she considers imperfect. Teachers describe her as a model student. She has headaches and insomnia.

**History.** Her older sibling received a prestigious scholarship. Hana says her parents never pressure her, but family conversation often centres on rankings. She fears disappointing teachers and being exposed as "actually average".

**Tasks.** Formulate perfectionism across cognitive, family, cultural, school, and identity dimensions. Distinguish excellence from compulsive checking. Design experiments in "good-enough" submission, sleep protection, and family feedback changes. Consider obsessive-compulsive assessment.

**Teaching point.** A school may reinforce the very pattern that harms the student because it produces desirable outputs.

## Case 8: The Friend Who Did Not Reply

**Presentation.** Bea, fifteen, has self-harmed twice after friendship conflicts and sends repeated messages seeking reassurance. She describes friends as either perfect or cruel. Adults use the phrase "attention-seeking".

**Safety note.** This fictional case requires a full current risk assessment in any teaching discussion. Do not treat the vignette as sufficient information for real intervention.

**Tasks.** Apply IPT-A, mentalization, emotion-focused, attachment, and DBT-informed lenses. Identify where each lens helps or risks blame. Develop a crisis plan, family communication plan, and school response that avoids both punitive withdrawal and unlimited reassurance. Examine staff language and team splitting.

**Teaching point.** Behaviour described as attention-seeking is often attachment- and safety-seeking expressed under high arousal. The task is to make safer bids more effective, not to make need disappear.

## Case 9: The Quiet Ward of One

**Presentation.** Kofi, eleven, recently placed in foster care, is polite with adults and refuses school. He hoards food in his room, wets the bed, and becomes distressed when routines change. He has had three schools in two years.

**Tasks.** Use trauma-informed, attachment, developmental, ecological, and educational perspectives. Prioritise placement stability, predictable routines, school transition, medical and continence assessment without shame, and carer support. Decide what trauma-specific work, if any, is appropriate now.

**Teaching point.** Stabilisation is treatment, not waiting. A new therapy relationship cannot compensate for repeated environmental discontinuity unless the system also changes.

## Case 10: The Joke That Travelled

**Presentation.** A private joke image of Priya, fourteen, is altered and shared beyond the intended group. She refuses school, deletes accounts, then creates new anonymous accounts at night. Some students claim she "sent it first" and should expect consequences.

**Tasks.** Address safeguarding, online exploitation or bullying pathways, school response, family support, sleep, peer bystanders, and the distinction between consensual sharing and non-consensual redistribution. Avoid victim blaming. Plan graded school return and digital safety without demanding that Priya educate the school.

**Teaching point.** Digital harm is interpersonal, institutional, and embodied. Individual coping cannot substitute for platform, peer, and school action.

## Case 11: The Eldest Daughter's Timetable

Ruth, seventeen, organises medication, translates appointments, cooks for siblings, and supports her mother after night shifts. School calls her exceptional. She has stopped applying for courses away from home and says she has no preferences “as long as everyone is okay”.

Students should examine young caring, gendered expectations, family resources, cultural values, identity foreclosure, and service responsibility. A successful plan reduces extracted resilience rather than praising it. Include respite, young-carer assessment, adult interpreting, school flexibility, and supported exploration of futures. Ask what Ruth might want if wanting did not feel disloyal.

## Case 12: The Name on the Register

Alex, fifteen, uses a chosen name with friends and a different name at home. A school system automatically prints the legal name on a classroom list projected to the class. Alex leaves school and refuses to return until “adults stop making it a thing”.

Students should address administrative safety, confidentiality, bullying, family disclosure planning, staff training, record systems, and the difference between affirmation and forced visibility. The first intervention may be technical: change the list. Identity support fails when databases keep outing the student.

## Case 13: The House with Two Alarms

Fatima, thirteen, lives with her mother and stepfather. She becomes aggressive after school but is calm in lessons. Her mother says Fatima saves all anger for home. Teachers suspect the school report is exaggerated.

Students should avoid assuming home is either safe or unsafe based on one setting. Explore after-school restraint collapse, sensory load, sleep, family conflict, possible neurodevelopmental needs, and the mother's experience. Design observation across settings and a decompression routine. Assess safeguarding directly and separately.

## Case 14: The Boy Who Coached Himself

Diego, twelve, is an talented footballer who panics before matches. He watches elite athletes online and compares his body and performance. His coach praises “mental toughness” and benches players who show distress.

Students should integrate anxiety, body image, social learning, sport culture, parent and coach behaviour, and performance systems. Intervention may include graded exposure, coping models, coach consultation, sleep and nutrition referral, and redefining courage as flexible participation rather than suppression.

## Case 15: A New Country, an Old Profession

Samira's mother was a pharmacist before migration and now cleans at night. Samira interprets for her and feels both proud and trapped. Her mother grieves status loss but tells school she is “fine”.

Students should explore intergenerational acculturation, credential recognition, grief, language brokering, class change, racism, and family pride. The counsellor should not turn structural loss into a communication exercise alone. Community, employment, language, and credential-navigation resources are part of care.

## Case 16: The Student Council President

Nia, seventeen, leads campaigns, supports friends, and achieves top grades. She presents after fainting and admitting she eats very little before school events. She says asking for help would make her “a fraud”.

Students should arrange medical assessment, explore perfectionism, identity, social reward for selflessness, family and school expectations, and possible eating disorder pathways. The plan must not rely solely on insight. High public functioning can conceal high private risk.

## Integrative Questions for Cases 11–16

1. Which adult system benefits from the young person's current coping?
2. Which strength is being overused until it becomes a burden?
3. What would change if the institution altered one routine?
4. What identity story is available to the young person besides patient, carer, problem, or high achiever?
5. Which professional is coordinating, and does the young person know?
6. What cultural assumptions did your first formulation make?
7. What outcome would the young person call success?
8. What would make you stop, refer, or escalate?

## Case 17: The Revolving Door

**Presentation.** Mara, sixteen, has had four counsellors in eighteen months due to staff turnover, placement change, and service age thresholds. She attends once, tells a detailed trauma history quickly, then does not return. Records describe “poor engagement”.

**Formulation prompts.** Consider attachment, epistemic trust, service discontinuity, assessment fatigue, and the possibility that rapid disclosure is a test or a learned attempt to secure help before disappearance. The system may be training the pattern it condemns.

**Plan prompts.** Offer continuity guarantees that are actually deliverable, a named backup practitioner, a concise shared formulation so Mara does not restart from zero, control over pacing, and an explicit repair

route after missed sessions. Measure trust and participation, not only symptoms.

## Case 18: The Examination Countdown

**Presentation.** Eli, seventeen, studies at a high-performing school. He has panic attacks, insomnia, and thoughts that failure would "prove" he has wasted his parents' sacrifices. His parents immigrated and work long hours. They say they never pressure him.

**Formulation prompts.** Examine explicit and implicit expectations, perfectionism, family sacrifice narratives, school comparison systems, sleep, caffeine, identity, and possible depression or obsessive-compulsive symptoms. Avoid reducing parental love to pressure or Eli's ambition to pathology.

**Plan prompts.** Medical and risk assessment; sleep protection; examination planning; graded exposure to imperfect work; family conversation about love, worth, and disappointment; school advocacy on mock examination feedback; career exploration that widens the definition of a viable future.

## Case 19: The Sibling Who Became Security

**Presentation.** Owen, ten, follows his seven-year-old autistic brother everywhere at school and intervenes in peer conflict. Staff praise him. Owen has stomach pain and refuses clubs because "he might need me".

**Formulation prompts.** Sibling subsystem, parentification, school reliance on a child for supervision, anxiety, disability support gaps, and family stress.

**Plan prompts.** Increase adult supervision and sibling support; give Owen protected peer time; create a plan for his brother that does not depend on Owen; family session validating both children's needs. Do not solve an institutional staffing issue through a child's vigilance.

## Case 20: The Last Ten Minutes

**Presentation.** In nearly every counselling session, Hana, fifteen, becomes distressed in the final ten minutes and asks to stay longer. The counsellor repeatedly overruns, then feels resentful. Hana says sessions are the only place adults do not rush her.

**Formulation prompts.** Attachment, endings, time boundaries, possible testing, session pacing, unprocessed material introduced too late, and practitioner countertransference.

**Plan prompts.** Structure sessions with a closing phase, summarise earlier, agree what can be carried safely, plan post-session support, and address the pattern collaboratively. Reliability includes ending on time with warmth, not only extending in crisis.

## Model Integrative Plan Skeleton

**Young person's stated goal:** _____

**Safety and consent position:** _____

**One-paragraph formulation:** _____

**First stabilising changes (system and individual):** _____

**Primary therapeutic focus and theory:** _____

**Family/school/service actions:** _____

**Skills or mastery sequence:** _____

**What will not be attempted yet, and why:** _____

**Measures and youth-defined outcomes:** _____

**Review date and decision rules:** _____

**Relapse/ending/transfer plan:** _____

## Portfolio Marking Guidance

High-quality portfolio work:

- distinguishes facts, reports, inferences, and unknowns;
- centres the young person's voice without treating it as the only evidence;
- integrates development, culture, family, school, and material context;
- names risk and protective processes dynamically;
- sequences interventions rather than collecting techniques;
- assigns institutional responsibilities to institutions;
- protects confidentiality and obtains meaningful assent;
- identifies practitioner limits and referral points;
- states outcomes that matter in ordinary life;
- revises formulation when evidence changes.

Weak work diagnoses from a paragraph, stereotypes culture, blames parents or the young person, lists brand-name therapies, ignores safeguarding, or proposes individual coping for structural harm.

## Additional Short Cases for Timed Formulation Practice

Use one case in a thirty-minute exercise: five minutes to identify facts and unknowns, ten minutes for a five-part formulation, five minutes for competing theory readings, five minutes for a sequenced plan, and five minutes for ethical and outcome review.

### Case 21: The Unopened Envelope

A seventeen-year-old avoids opening college messages after one rejection and says the future has “already decided”. Focus areas include depression, avoidance, shame, family expectations, academic triage. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 22: The Seat by the Door

A twelve-year-old insists on sitting near exits and becomes distressed when classroom seating changes. Focus areas include anxiety, safety behaviour, sensory context, teacher consultation. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 23: The Captain Who Quit

A talented team captain leaves sport after panic symptoms and online criticism of a missed goal. Focus areas include performance anxiety, social learning, body image, coaching culture. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 24: The Brother on Guard

A ten-year-old monitors his disabled sibling at school and refuses clubs. Focus areas include sibling subsystem, parentification, school support gaps. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 25: The Group Chat Election

A student is removed from a friendship group chat and begins checking messages through another pupil’s account. Focus areas include belonging, digital surveillance, peer dispute, school safety. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 26: The Quiet High Achiever

A student with excellent grades cries nightly, checks work repeatedly, and cannot submit “imperfect” assignments. Focus areas include perfectionism, OCD assessment, family and school reinforcement. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 27: The New Pronunciation

A migrant student stops speaking in class after classmates imitate her accent. Focus areas include language, racism, shame, interpreter and school action. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 28: The Sunday Bag

A child packs a bag repeatedly before contact visits and becomes ill on transition mornings. Focus areas include separated families, transitions, loyalty, predictable planning. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 29: The Student Who Sleeps in Class

A fifteen-year-old falls asleep in morning lessons after caring for a parent at night. Focus areas include young caring, sleep, poverty, school flexibility. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 30: The Locker Room Detour

A student takes a long route to avoid changing rooms and is sanctioned for lateness. Focus areas include bullying, body privacy, facilities, safeguarding. Students should identify what must be assessed before

intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 31: The Apology Tour

After every argument, a young person sends long apologies to friends and panics until each replies. Focus areas include reassurance, shame, mentalizing, peer communication. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 32: The Empty Chair at Dinner

A bereaved adolescent leaves the table when a grandparent's former seat is discussed. Focus areas include grief, family rituals, school support, meaning. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 33: The Second Phone

A teenager keeps a hidden phone after inconsistent device rules across two homes. Focus areas include autonomy, transitions, parental conflict, digital safety. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 34: The Girl Who Translated Everything

A fifteen-year-old interprets medical letters and misses her own activities. Focus areas include language brokering, parentification, professional interpreters. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond

symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 35: The Boy Who Laughed First

A student jokes whenever corrected and is described as having no respect. Focus areas include shame, peer audience, discipline, restorative approaches. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 36: The Missing Lunch

A student avoids the dining hall because queues are noisy and seating is socially unpredictable. Focus areas include sensory load, anxiety, school design, graded participation. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 37: The Last-Minute Genius

A student produces strong work only under extreme deadline pressure and sleeps poorly. Focus areas include procrastination, perfectionism, reward timing, study systems. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 38: The Helper Who Never Asked

A student supports every friend in crisis but conceals her own self-harm urges. Focus areas include identity, help seeking, peer burden, risk assessment. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist

diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 39: The Protest at the Gate

A student refuses to enter school after a search policy they experience as discriminatory. Focus areas include institutional trust, rights, discrimination, consultation. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

## Case 40: The Bedroom Studio

An adolescent spends nights making music online, gains supportive collaborators, and falls asleep at school. Focus areas include digital creativity, belonging, sleep, family negotiation. Students should identify what must be assessed before intervention, which adult or institutional response may be maintaining the difficulty, what the young person identifies as a good outcome, and what first step is small enough to test. Include one outcome beyond symptom reduction—participation, recovery time, relationship quality, sleep, belonging, or help seeking—and state what information would lead you to refer or escalate. The strongest responses will resist diagnosing from the vignette and will show how culture, development, family, school, and material context alter the plan.

CHAPTER APPENDIX C

# Theory Comparison Matrices

Side-by-side comparison of all theories covered in the course.

## C1. How to Use These Matrices

These matrices are revision and supervision tools, not league tables. A theory should be judged against a specific young person, problem, context, goal, evidence base, practitioner competence, and risk profile. “Best” in one row may be irrelevant or unsafe in another.

## Matrix 1: Foundational Orientation

| Theory/family | Person viewed as | Main source of difficulty | Primary change process | Counsellor role |
|---|---|---|---|---|
| Person-centred | Self-directing person under conditions of worth | Incongruence and denied experience | Empathy, congruence, unconditional positive regard | Facilitative witness |
| Existential | Freedom-bearing person facing finitude and choice | Avoidance of meaning, freedom, isolation, mortality | Authentic encounter and responsibility | Philosophical companion |
| Gestalt | Organism in contact with environment | Interrupted awareness and contact | Present-focused awareness and experiment | Active facilitator |
| Cognitive therapy | Information processor shaped by schemas | Maladaptive appraisal and belief | Testing and restructuring cognition and behaviour | Collaborative empiricist |
| REBT | Thinking and evaluating agent | Rigid demanding beliefs | Disputation and new philosophy | Direct teacher-coach |
| Behaviour therapy | Learner in environments | Conditioning, reinforcement, avoidance, skill deficits | New learning and contingency change | Designer and coach |
| Family systems | Person in relational systems | Interaction patterns, structure, boundaries, sequences | System reorganisation | System consultant |
| Attachment-based | Relational being shaped by care | Insecure expectations and regulation difficulties | Responsive relationships and repair | Secure-base figure/coach |
| Narrative | Author within discourses | Problem-saturated stories and power | Externalisation and re-authoring | Co-editor and witness |
| Solution-focused | Resourceful actor | Problem talk and overlooked exceptions | Exceptions, goals, small steps | Curious questioner |
| Trauma-informed | Adaptive survivor in context | Threat responses and unsafe conditions | Safety, stabilisation, paced processing | Safety partner and coordinator |
| Mentalization-based | Mind interpreting minds | Mentalizing collapse under arousal | Curiosity, affect reflection, epistemic trust | Not-knowing co-thinker |

## Matrix 2: Adolescent Suitability and Cautions

| Approach | Often useful when | Adapt for adolescents by | Use caution when |
|---|---|---|---|
| Person-centred | Shame, low agency, need for trust | Add structure, family/school context, active safeguarding | Risk requires directive coordination |
| Existential | Identity, grief, choice, meaning | Use concrete language and developmental framing | Acute crisis or severe concrete thinking dominates |
| Gestalt | Avoided emotion, contact interruption | Short experiments, choice, play/media | Flooding, dissociation, body shame |
| CBT | Anxiety, depression, panic, specific beliefs | Examples, family/school practice, shorter tasks | Literacy, neurodivergence, unsafe environments not adapted |
| REBT | Rigid self-demands and frustration intolerance | Humour carefully, collaborative language | It sounds blaming or argumentative |
| Behavioural | Observable avoidance, skills, routines | Youth-chosen reinforcers, adults change contingencies | Behaviour is communication of danger or unmet need |
| Family systems | Conflict and maintaining sequences | Protect youth voice and individual space | Abuse or coercive control makes joint work unsafe |
| Attachment | Relational insecurity and carer capacity | Work through caregivers and trusted adults | Placement instability overwhelms dyadic work |
| Narrative | Shame and imposed labels | Drawings, letters, groups, school documents | Material danger is story-treated instead of changed |
| Solution-focused | Engagement and practical goals | Concrete scales and youth language | Pain is bypassed or risk minimised |
| EMDR/trauma-focused | Discrete processed trauma memories | Preparation, caregiver, short sets, stabilisation | Ongoing threat, instability, untrained delivery |
| MBT | Self-harm, relational crises, affect instability | Family/team mentalizing and crisis plans | High arousal, unsafe family interaction, jargon |
| IPT-A | Depression linked to grief/disputes/transitions | Family and school involvement, social media context | Severe risk or non-interpersonal mechanisms dominate |
| MI | Ambivalence and mandated contact | Autonomy transparency and youth values | Used manipulatively to manufacture consent |

## Matrix 3: Presenting Pattern to First Questions

| Presentation | Ask first | Possible mechanisms | Do not miss |
|---|---|---|---|
| School refusal | Which lessons, routes, people, times? What happens after staying home? | Avoidance, bullying, panic, learning gap, family accommodation, sleep, transport | Safeguarding, medical issues, SEND/access, unsafe school conditions |

| Presentation | Ask first | Possible mechanisms | Do not miss |
|---|---|---|---|
| Aggression | What precedes escalation? What does it stop or obtain? | Escape, status, pain, impulsivity, trauma state, communication limits | Weapon risk, abuse, neurodevelopment, mood disorder, adult escalation |
| Self-harm | Current intent, plans, means, functions, supports | Affect regulation, shame, interpersonal crisis, trauma, depression | Medical care, coercion, online harm, access to means |
| Low mood | Sleep, activity, reward, relationships, loss, future | Withdrawal, anhedonia, grief, conflict, illness, isolation | Suicide risk, bipolar features, substances, eating, thyroid/medical review |
| Aggression at home only | What happens after school? Who sees what? | Restraint collapse, family sequence, sensory load, safety cues | Domestic abuse, carer exhaustion, sibling risk |
| Silence/non-speaking | Where, with whom, since when, what helps? | Anxiety, language, trauma state, communication difference, protest | Selective mutism assessment, hearing/language, coercion |
| Eating restriction | Medical status, history, body image, control, family meals | Anxiety, sensory issues, eating disorder, GI illness, trauma | Urgent medical risk, growth, electrolytes per medical advice |
| Substance use | Function, pattern, peers, supply, consequences | Coping, belonging, dependence, trauma, exploitation | Withdrawal, county-lines/exploitation, polysubstance risk |
| Perfectionism | What error means, checking, sleep, feedback | Shame, cognitive rigidity, family/school contingencies, OCD | Eating, self-harm, obsessive-compulsive disorder, burnout |
| Friendship crisis | Sequence, interpretations, digital context, support network | Mentalizing collapse, dispute, exclusion, identity, bullying | Image sharing, coercion, suicidal response |

## Matrix 4: Mechanism-to-Intervention Guide

| If the main mechanism is… | Consider… | Pair with… |
|---|---|---|
| Avoidance negatively reinforced | Graded exposure and response prevention | Family accommodation reduction, school plan |
| Low reward and withdrawal | Behavioural activation | Sleep, social connection, academic triage |
| Low mastery/self-efficacy | Graded tasks, coping models, feedback | Social cognitive planning |
| Skill deficit | Instruction, modelling, rehearsal, homework | Generalisation in real settings |
| Escalating family sequence | Parent coaching, communication analysis | School alignment, safety plan |

| If the main mechanism is… | Consider… | Pair with… |
|---|---|---|
| Shame-based identity story | Narrative externalisation | Affirmative support and system change |
| Mentalizing collapse | Stop–rewind–explore, affect naming | Crisis plan and team consistency |
| High autonomic arousal | Grounding, orienting, movement, sleep | Trauma phase assessment |
| Interpersonal dispute | IPT-A communication and decision analysis | Family/school negotiation |
| Ambivalence | Motivational interviewing | Practical barrier removal |
| Grief | Grief support, IPT-A, narrative and family rituals chosen by family | School flexibility and memorial boundaries |
| Structural harm | Advocacy, consultation, safeguarding, service coordination | Individual coping only as adjunct |

## Matrix 5: Evidence and Implementation Questions

For every intervention selected, answer:

1. For which age group and problem was it tested?
2. Were young people like this one excluded?
3. What training and supervision are required?
4. How many sessions and what setting?
5. What adverse effects or burdens exist?
6. What outcome will be measured, by whom, and when?
7. What family, cultural, linguistic, disability, or digital adaptations are needed?
8. What would make us stop or refer?

A model is not implemented merely because its name appears in a plan. Fidelity, dose, sequencing, and context are part of evidence-based practice.

## Matrix 6: Session Focus Selector

| If the young person arrives… | First priority may be… |
|---|---|
| In immediate danger | Safeguarding, medical or crisis action |
| Highly aroused | Regulation, safety, reduced demand |
| Blank or dissociated | Orientation, choice, gentle pacing |
| With an urgent practical problem | Problem-solving and advocacy |
| Ready to review homework/exposure | Specific feedback and mastery attribution |
| After a family rupture | Repair, sequence analysis, safety |

| If the young person arrives… | First priority may be… |
|---|---|
| Before a known transition | Rehearsal, supports, contingency planning |
| At planned ending | Consolidation, relapse plan, transfer of supports |

CHAPTER APPENDIX D

# Glossary of Terms

A comprehensive glossary of theory, practice, and developmental terminology.

This glossary prioritises working definitions for students. It does not replace diagnostic manuals, statutes, or specialist texts.

## Key Terms at a Glance

- **Abstinence violation effect** — A lapse after a period of abstinence interpreted as total failure, increasing risk of full relapse.
- **Accommodation (family)** — Family changes that reduce a young person's immediate distress but may maintain avoidance or symptoms.
- **Acting out** — Expressing conflict or feeling through action rather than words or reflection.
- **Active listening** — Listening that communicates attention through posture, minimal encouragers, reflection, and summary.
- **Activity scheduling** — Planning valued, mastery, social, and restorative activities as part of treatment.
- **Acute risk** — Immediate or near-term danger requiring urgent action.
- **Adaptive behaviour** — Behaviour that helps a person manage demands in a context, even if it has longer-term costs.
- **Adolescence** — Developmental period involving puberty, identity, autonomy, peer belonging, and transition toward adult roles; boundaries vary.
- **Adverse childhood experiences (ACEs)** — Potentially traumatic events or conditions in childhood associated at population level with later health risks.
- **Affect** — Observable expression of emotion.
- **Affect regulation** — Processes by which emotional arousal is noticed, tolerated, modulated, and expressed.
- **Affirmation** — In motivational interviewing, specific recognition of strengths, values, effort, or autonomy.
- **Agency** — Capacity to initiate action and influence one's life within real constraints.
- **Agenda mapping** — Collaboratively identifying and prioritising topics for change.
- **Agoraphobia** — Anxiety and avoidance concerning places or situations from which escape may be difficult or help unavailable during panic-like symptoms.
- **Alliance** — Collaborative therapeutic relationship involving bond, goals, and tasks.
- **Ambivalence** — Simultaneous reasons for and against a change or course of action.
- **Amplification** — Gestalt or reflective technique of exploring an experience more fully; must be used without flooding.
- **Anhedonia** — Reduced ability to experience pleasure or interest.
- **Antecedent** — Event or condition occurring before a behaviour in functional analysis.

## Key Terms at a Glance (continued)

- **Anxiety** — Anticipatory fear response involving cognition, physiology, and avoidance or preparation.
- **Applied behaviour analysis (ABA)** — Systematic application of learning principles to socially significant behaviour, including functional assessment.
- **Approach behaviour** — Movement toward a feared, valued, or needed stimulus or goal.
- **Arousal** — Physiological and psychological activation.
- **Assertiveness** — Communicating needs and boundaries while respecting others.
- **Assessment** — Systematic gathering and interpretation of information for decisions.
- **Assent** — A young person's affirmative agreement to participate, distinct from legal consent.
- **Assimilation (cognitive)** — Interpreting new experience through existing schemas; in cultural contexts it may have different meanings and should be defined locally.
- **Attachment** — Enduring relational system involving proximity, safe haven, secure base, and expectations of care.
- **Attachment figure** — Person used for comfort, protection, and exploration support.
- **Attending** — Practitioner behaviours that communicate presence and attention.
- **Attention-maintained behaviour** — Behaviour maintained partly by social responses it produces.
- **Attribution** — Explanation assigned to an event or behaviour.
- **Automatic thought** — Rapid evaluative thought arising in a situation.
- **Autonomic nervous system** — Nervous system divisions involved in arousal, digestion, cardiovascular and threat responses.
- **Autonomy** — Self-direction consistent with development and safety.
- **Avoidance** — Behaviour that prevents contact with feared, painful, or demanding stimuli and is often negatively reinforced.
- **Baseline** — Measurement before intervention used for comparison.
- **Behaviour** — Observable action; some traditions also include private events defined functionally.
- **Behavioural activation** — Treatment increasing rewarding, mastery, social, and value-based activity to interrupt depressive withdrawal.
- **Behavioural experiment** — Planned test of a belief or prediction through action.
- **Belonging** — Experience of acceptance, recognition, and participation in a group or setting.
- **Bibliotherapy** — Guided use of reading materials as an adjunct to care.
- **Bilateral stimulation** — Alternating sensory stimulation used in EMDR protocols.

## Key Terms at a Glance (continued)

- **Biopsychosocial model** — Framework considering biological, psychological, and social factors together.
- **Body scan** — Attention moved systematically through bodily sensations; requires consent and trauma sensitivity.
- **Boundaries** — Limits and expectations governing roles, contact, privacy, and responsibility.
- **Brief therapy** — Time-conscious treatment organised around focal goals and efficient change.
- **Broaching** — Inviting discussion of identity, culture, difference, and power.
- **Bullying** — Repeated harmful behaviour involving power imbalance, including online forms.
- **Burnout** — Occupational exhaustion, cynicism, and reduced efficacy associated with chronic work stress.
- **Case conceptualisation** — Collaborative formulation explaining development and maintenance of difficulties and guiding treatment.
- **Case management** — Coordination of services, referrals, and practical supports.
- **Catharsis** — Emotional release; not by itself sufficient evidence of therapeutic change.
- **Change talk** — Client language expressing desire, ability, reasons, need, commitment, activation, or steps toward change.
- **Chosen family** — Self-selected network providing kinship-like support.
- **Circadian rhythm** — Approximately daily biological cycle affecting sleep, alertness, hormones, and mood.
- **Classical conditioning** — Learning through association between stimuli.
- **Clinical formulation** — See case conceptualisation.
- **Clinical significance** — Practical importance of a change beyond statistical significance.
- **Co-regulation** — Regulation of arousal through relationship with another person.
- **Cognition** — Mental processes including attention, memory, interpretation, belief, and problem-solving.
- **Cognitive defusion** — ACT process of noticing thoughts as thoughts rather than literal commands or facts.
- **Cognitive distortion** — Biased or inaccurate pattern of interpretation; best used descriptively rather than as blame.
- **Cognitive restructuring** — Identifying, testing, and revising unhelpful appraisals or beliefs.
- **Cognitive triad** — Beck's negative view of self, world/experience, and future in depression.
- **Cohesion** — In groups or families, sense of belonging and commitment.

### Key Terms at a Glance (continued)

- **Collaborative empiricism** — CBT stance in which client and practitioner test ideas together.
- **Collective efficacy** — Shared belief that a group can organise and act effectively.
- **Common factors** — Therapeutic elements shared across models, including alliance, empathy, expectancy, goals, and feedback.
- **Communication analysis** — Detailed reconstruction of an interpersonal exchange.
- **Comorbidity** — Presence of more than one condition or difficulty at the same time.
- **Compassion** — Concern for suffering with motivation to help; in MI, prioritising client welfare.
- **Competence** — Knowledge, skill, judgement, and ethical capacity to perform a professional task.
- **Conditioned response** — Learned response to a previously neutral stimulus after association.
- **Confidentiality** — Ethical and legal duty to protect private information within stated limits.
- **Conflict (interpersonal)** — Dispute arising from incompatible goals, expectations, needs, or interpretations.
- **Congruence** — Practitioner genuineness and consistency between experience and communication.
- **Consent** — Informed, voluntary agreement by a person with legal authority and capacity for the decision.
- **Consequence** — Event following behaviour that may alter its future likelihood.
- **Containment** — Relational process by which overwhelming affect is received, tolerated, and returned in manageable form.
- **Contingency** — Relationship between behaviour and its consequences.
- **Contingency management** — Planned alteration of reinforcement and consequences in a setting.
- **Coping** — Cognitive, behavioural, emotional, relational, or spiritual-neutral strategies used to manage demands; may be helpful or costly.
- **Core belief/schema** — Deeply held general belief about self, others, or world.
- **Corrective experience** — New relational or behavioural experience that updates expectations.
- **Countertransference** — Practitioner emotional and relational responses in treatment, requiring reflection.
- **Crisis** — Situation in which demands exceed current coping or safety resources.
- **Crisis plan** — Agreed steps, contacts, and supports for periods of elevated risk.
- **Cultural formulation** — Assessment of cultural identity, explanations, context, and clinician-client differences.

## Key Terms at a Glance (continued)

- **Cultural humility** — Lifelong self-critique, openness, and accountability regarding culture and power.
- **Culture** — Shared and contested patterns of meaning, practice, identity, language, values, and institutions.
- **Defence mechanism** — Psychological strategy reducing awareness of painful affect or conflict.
- **De-escalation** — Actions that reduce arousal, threat, and likelihood of harm.
- **Deliberate self-harm** — Self-injury or self-poisoning, regardless of suicidal intent; local terminology should be used carefully.
- **Denial** — Defence involving refusal to acknowledge aspects of reality.
- **Dependence** — Reliance on a person, substance, or behaviour; developmental dependence is normal in childhood.
- **Desensitisation** — Reduced emotional response after repeated or graded contact.
- **Developmental cascade** — Effects spreading across domains and developmental periods.
- **Developmental trauma** — Impact of chronic, interpersonal adversity during development; not a stand-alone diagnosis in major manuals.
- **Diagnosis** — Classification of symptoms and patterns for communication, planning, and eligibility.
- **Dialectics** — Holding apparently opposing truths together, such as acceptance and change.
- **Differentiation of self** — Capacity to maintain self-definition while remaining emotionally connected.
- **Digital safety** — Practices reducing online exploitation, bullying, privacy loss, and harmful contact.
- **Disenfranchised grief** — Grief not socially recognised or supported.
- **Disputation** — REBT process of questioning rigid or irrational beliefs.
- **Dissociation** — Disruption in consciousness, memory, identity, perception, or sense of self, often associated with overwhelm.
- **Distress tolerance** — Capacity to endure painful states without making them worse.
- **Dorsal vagal language** — Polyvagal-informed term for shutdown-related states; theoretical claims should be taught cautiously.
- **Double bind** — Contradictory communications with limited ability to comment or escape.
- **Dropout** — Ending treatment before agreed completion; meaning should be investigated rather than assumed.

### Key Terms at a Glance (continued)

- **Dual-systems model** — Adolescent model contrasting reward sensitivity with developing cognitive control.
- **Dyad** — Two-person relationship or session format.
- **Ecological systems theory** — Bronfenbrenner's model of nested developmental contexts.
- **Efficacy (self-)** — Belief in capability for a specific task in a specific context.
- **Ego states** — In transactional analysis, Parent, Adult, and Child patterns of feeling, thinking, and behaving.
- **Emotion coaching** — Helping a person notice, name, validate, regulate, and act on emotion.
- **Emotion-focused therapy** — Treatment working with primary and secondary emotions and unmet needs.
- **Emotional cutoff** — Distancing from family relationships to manage unresolved attachment intensity.
- **Empathy** — Understanding and communicating another person's experience from their frame of reference.
- **Empowerment** — Increasing agency, voice, resources, and control.
- **Enactment** — Relational pattern expressed in action, including in therapy.
- **Engagement** — Process of establishing participation and working relationship.
- **Epistemic trust** — Openness to learning from another as a trustworthy source of social knowledge.
- **Escape-maintained behaviour** — Behaviour maintained by removing or postponing demands or aversive conditions.
- **Ethics** — Principles and standards governing professional conduct.
- **Evidence-based practice** — Integration of best research evidence, clinical expertise, client preferences, and context.
- **Exception** — In solution-focused work, a time when the problem is absent, weaker, or handled differently.
- **Exposure** — Planned contact with feared stimuli or memories under conditions that permit new learning.
- **Externalisation** — Narrative practice separating person from problem so the problem's influence can be examined.
- **Extinction** — Reduction of behaviour when reinforcement is withheld; may involve an initial burst.

## Key Terms at a Glance (continued)

- **Family projection process** — Transmission of parental anxiety or unresolved issues onto a child.
- **Family systems** — Patterns of interaction, structure, boundaries, and feedback in families.
- **Feedback-informed care** — Routine use of client outcome and alliance feedback to adjust treatment.
- **Feminist counselling** — Practice analysing gender, power, social context, and empowerment.
- **Fidelity** — Degree to which an intervention is delivered as intended.
- **Fight-or-flight** — Mobilising threat responses preparing confrontation or escape.
- **Flashback** — Intrusive re-experiencing of traumatic material as if occurring now.
- **Flooding** — Intense exposure approach; generally requires caution and is not first-line for most youth presentations.
- **Formulation** — See case conceptualisation.
- **Freeze** — Threat response involving immobility or shutdown.
- **Functional analysis** — Assessment of antecedents, behaviours, consequences, and maintaining functions.
- **Functional behaviour assessment** — Educational and clinical assessment of behaviour purpose and context.
- **Gender dysphoria** — Clinically significant distress associated with incongruence between experienced gender and assigned sex, as defined in diagnostic systems.
- **Gender expression** — External communication of gender through appearance, behaviour, voice, and roles.
- **Gender identity** — Internal sense of one's gender.
- **Generalisation** — Transfer of learning or behaviour change across settings, people, and time.
- **Genogram** — Family diagram showing relationships, patterns, events, and generations.
- **Goal** — Desired outcome guiding treatment; should be meaningful and reviewable.
- **Grief** — Response to loss, including emotional, bodily, cognitive, social, and behavioural dimensions.
- **Grounding** — Techniques orienting attention to present sensory reality to manage arousal or dissociation.
- **Group cohesion** — Members' sense of belonging, trust, and commitment in a group.

## Key Terms at a Glance

- **Habit reversal** — Behavioural procedure involving awareness, competing responses, and practice for repetitive behaviours.
- **Habituation** — Decreased response after repeated stimulation; not the only mechanism proposed for exposure.
- **Harm reduction** — Practical strategies reducing negative consequences of risky behaviour without requiring immediate abstinence as the only goal.
- **Hierarchy (exposure)** — Ordered list of feared situations from manageable to most difficult.
- **Hierarchy (family)** — Organisation of authority and responsibility across generations and subsystems.
- **Homeostasis** — System tendency to maintain stability, sometimes including symptom patterns.
- **Homework** — Between-session practice agreed in therapy.
- **Hyperarousal** — Excessive activation involving vigilance, startle, irritability, tension, or panic.
- **Hypoarousal** — Reduced activation involving numbness, shutdown, slowed response, or dissociation.
- **Hypermentalizing** — Overconfident, elaborate interpretation of mental states from limited evidence.
- **Hypomentalizing** — Limited reflection on mental states.
- **Identity** — Organised sense of self across time, roles, values, groups, body, and future.
- **Idiographic measure** — Measure tailored to an individual's goals or target behaviours.
- **Immediacy** — Practitioner use of here-and-now relationship experience.
- **Impulsivity** — Action with limited forethought; may reflect development, arousal, neurodivergence, substances, or context.
- **Incongruence** — Rogers' discrepancy between self-concept and experience.
- **Individuation** — Development of distinct identity and autonomy within relationships.
- **Informed consent** — Consent based on understandable information about nature, benefits, risks, alternatives, and rights.
- **Inhibitory learning** — Exposure account in which new safety learning competes with threat associations.
- **Insight** — Understanding of patterns or meanings; useful but not always sufficient for change.
- **Internal object** — Mental representation of self in relation to another, charged with affect.
- **Internal working model** — Expectations about self, others, and relationships derived from attachment experience.

## Key Terms at a Glance (continued)

- **Internalised stigma** — Applying negative social stereotypes to oneself.
- **Interpersonal inventory** — IPT assessment of important relationships and problem areas.
- **Interpersonal psychotherapy (IPT)** — Time-limited treatment targeting grief, disputes, transitions, and interpersonal deficits.
- **Intersectionality** — Framework analysing interacting identities and systems of power.
- **Intervention** — Intentional action intended to improve health, functioning, learning, or wellbeing.
- **Interview (motivational)** — See motivational interviewing.
- **Intimate partner violence** — Physical, sexual, psychological, economic, or coercive harm within intimate relationships; adolescent dating violence requires safeguarding.
- **Introjection** — Internalising another's attitudes or projections as part of self-experience.
- **Invalidation** — Communication that emotions or perceptions are wrong, excessive, or unacceptable.
- **Irrational belief (REBT)** — Rigid, illogical, or absolutist belief associated with disturbance in Ellis's model.
- **Joining** — Family therapy process of entering the family's style to build alliance and influence.
- **Joint session** — Meeting involving young person and family members or professionals together.
- **Learned helplessness** — Expectation that action cannot influence outcomes after uncontrollable aversive experience.
- **Learning theory** — Accounts of behaviour change through conditioning, observation, cognition, and environment.
- **Least restrictive intervention** — Effective option imposing the least limitation on rights and autonomy.
- **Limit setting** — Clear, proportionate boundaries for safety and therapeutic structure.
- **Listening (reflective)** — Responding that conveys understanding of content, feeling, meaning, or ambivalence.
- **Locus of control** — Beliefs about whether outcomes are mainly controlled by oneself or external forces.
- **Loss** — Loss of a person, role, place, capacity, relationship, status, or imagined future.
- **Maintenance** — Preservation of gains after active treatment.
- **Maladaptive** — Pattern that creates harm or impairment in context, even if originally protective.
- **Mandated treatment** — Treatment required by court, school, safeguarding, or another authority.

### Key Terms at a Glance (continued)

- **Marked mirroring** — Caregiver reflection that represents a child's feeling without simply reproducing it overwhelmingly.
- **Mastery experience** — Successful performance providing evidence of capability.
- **Meaning-making** — Process of constructing significance from experience.
- **Mechanism of change** — Process through which intervention produces outcome.
- **Medical model** — Framework understanding problems primarily as disorders requiring diagnosis and treatment.
- **Mental health literacy** — Knowledge and beliefs aiding recognition, management, and prevention of mental distress.
- **Mentalization** — Understanding behaviour in terms of mental states.
- **Mentalization-based treatment (MBT)** — Treatment restoring mentalizing under arousal, especially in relational difficulties.
- **Mesosystem** — Connections among a young person's immediate settings.
- **Metacognition** — Awareness and regulation of one's own thinking.
- **Metacommunication** — Communication about communication or relationship.
- **Microsystem** — Immediate environment in which a person participates.
- **Mindfulness** — Present-moment attention with openness and non-judgement; clinical adaptations vary.
- **Minority stress** — Stress from stigma, discrimination, vigilance, concealment, and internalised negativity.
- **Miracle question** — Solution-focused question imagining life after the problem is solved.
- **Modelling** — Learning or teaching through observation of behaviour and consequences.
- **Mood monitoring** — Tracking mood and related variables over time.
- **Motivational interviewing (MI)** — Collaborative conversation strengthening a person's own motivation and commitment to change.
- **Multisystemic therapy** — Intensive family- and community-based intervention for serious adolescent antisocial behaviour.
- **Narrative therapy** — Practice separating persons from problems and developing preferred stories and unique outcomes.
- **Negative reinforcement** — Removal of an aversive condition that increases a behaviour.
- **Neurodevelopmental conditions** — Conditions affecting brain development, including autism, ADHD, intellectual disability, communication and specific learning disorders.

## Key Terms at a Glance (continued)

- **Neuroplasticity** — Nervous system's capacity to change with experience.
- **Nominal group technique** — Structured method for generating and prioritising ideas.
- **Nomothetic measure** — Standardised measure enabling comparison with groups or norms.
- **Non-suicidal self-injury** — Deliberate injury without suicidal intent; intent can be mixed and must be assessed.
- **Not-knowing stance** — Curious recognition that mental states cannot be known with certainty.
- **Object relations** — Theories of internalised self–other relationship patterns.
- **Observational learning** — Learning by attending to, retaining, reproducing, and being motivated by modelled behaviour.
- **Obsessions** — Intrusive, unwanted thoughts, images, or urges.
- **Compulsions** — Repetitive behaviours or mental acts performed to reduce distress or prevent feared events.
- **Operant conditioning** — Learning through consequences of behaviour.
- **Operational definition** — Behaviour defined in observable, measurable terms.
- **Oppositional behaviour** — Argumentative, defiant, or hostile behaviour; description is not an explanation.
- **Orienting** — Sensory process of locating oneself in present environment.
- **Outcome expectation** — Belief about likely consequences of behaviour.
- **Outcome monitoring** — Repeated measurement of progress and feedback during care.
- **Overgeneralisation** — Drawing broad conclusions from limited events.
- **Panic attack** — Sudden surge of intense fear or discomfort with physical and cognitive symptoms.
- **Parentification** — Child assuming adult practical or emotional roles.
- **Parent training** — Structured support helping caregivers use effective, safe, relationship-supporting practices.
- **Partnership (MI)** — Collaborative working relationship rather than expert imposition.
- **Parts work** — Clinical language for internal subpersonalities or states; requires care not to literalise beyond client meaning and evidence.
- **Peer support** — Support from people with shared experience, in informal or organised forms.
- **Pendulation** — Moving attention between activation and resource in somatic work.
- **Perpetuating factors** — Processes maintaining a problem.

## Key Terms at a Glance (continued)

- **Person-centred therapy** — Humanistic approach based on empathy, congruence, and unconditional positive regard.
- **Personalisation** — Interpreting events as personally targeted without sufficient evidence.
- **Phobia** — Marked, persistent fear and avoidance of specific objects or situations causing impairment.
- **Physiological state** — Bodily arousal, fatigue, pain, hunger, illness, and hormonal or medication effects influencing behaviour.
- **Planned ignoring** — Deliberate withdrawal of attention from a behaviour while maintaining safety and reinforcing alternatives.
- **Positive reinforcement** — Addition of a consequence that increases behaviour.
- **Post-traumatic stress disorder (PTSD)** — Disorder following trauma involving intrusion, avoidance, altered cognition/mood, and arousal under diagnostic criteria.
- **Power differential** — Unequal authority, knowledge, age, institutional status, or resources in a relationship.
- **Precipitating factors** — Events triggering onset or escalation.
- **Predisposing factors** — Earlier vulnerabilities and contexts increasing likelihood of difficulty.
- **Pretend mode** — Mental-state talk disconnected from affect and lived experience.
- **Prevention** — Action reducing incidence, severity, or impact of problems.
- **Primary emotion** — Immediate adaptive emotional response to a need or event.
- **Privilege** — Unearned advantage linked to social position or system design.
- **Problem area (IPT)** — Focal category: grief, dispute, role transition, or interpersonal deficit.
- **Problem-solving therapy** — Structured method for defining problems, generating options, choosing, implementing, and reviewing.
- **Procedural learning** — Learning embodied in patterns of action and response rather than explicit knowledge.
- **Projection** — Attributing disowned feelings or motives to another.
- **Projective identification** — Interpersonal process in which disowned states are evoked or enacted in another.
- **Promotive factor** — Resource associated with positive outcomes across risk levels.
- **Protective factor** — Resource reducing probability or impact of adverse outcomes.
- **Protocol** — Specified sequence and procedures for an intervention.
- **Proximal goal** — Near-term goal supporting a larger distal goal.

## Key Terms at a Glance (continued)

- **Psychic equivalence** — Treating feelings or beliefs as external facts.
- **Psychodynamic** — Approaches emphasising unconscious processes, conflict, defence, development, and relational repetition.
- **Psychoeducation** — Structured information about a condition, development, treatment, or coping.
- **Psychological flexibility** — ACT capacity to remain present and act according to values despite difficult internal experiences.
- **Psychosis** — Conditions involving impaired reality testing such as delusions, hallucinations, or disorganised thinking; requires specialist assessment.
- **Punishment** — Consequence reducing behaviour; positive punishment adds aversive stimulus, negative punishment removes valued stimulus.

## Key Terms at a Glance

- **Radical behaviourism** — Skinner's philosophy treating private events as behaviour subject to functional analysis.
- **Rapport** — Harmonious connection supporting communication.
- **Rational emotive behaviour therapy (REBT)** — Ellis's approach identifying and disputing rigid beliefs and building healthier philosophies.
- **Reactivity** — Change in behaviour because it is being observed or measured.
- **Re-authoring** — Narrative development of preferred accounts using unique outcomes and values.
- **Reciprocal determinism** — Bandura's bidirectional influence among person, behaviour, and environment.
- **Recovery** — Personal process of living a meaningful life with or without continuing symptoms; also clinical symptom improvement depending on context.
- **Reflection (listening)** — Response conveying understanding of a speaker's meaning or feeling.
- **Reflective functioning** — Capacity to reflect on mental states in self and others.
- **Reformulation** — Revised formulation based on new evidence or response to intervention.
- **Refractory problem** — Problem persisting despite apparently appropriate intervention; a prompt to reformulate, not blame.
- **Regression** — Return to earlier developmental modes under stress.
- **Reinforcement** — Consequence increasing future behaviour.
- **Reinforcement schedule** — Pattern governing when consequences follow behaviour.
- **Relapse** — Return of symptoms or behaviour after improvement.
- **Relapse prevention** — Planning for warning signs, high-risk contexts, coping, and help seeking.
- **Relational frame** — Context of relationships through which behaviour and meaning are understood.
- **Reliability** — Consistency of measurement or judgement.
- **Reparation** — Making amends or repairing relational harm.
- **Resilience** — Dynamic process of adaptation under adversity supported by individual and environmental systems.
- **Resistance** — Ambivalence, disagreement, avoidance, or interaction pattern opposing change; not simply a client defect.
- **Resourcefulness** — Ability to identify and use internal and external resources.
- **Response cost** — Removal of a valued stimulus following behaviour.

## Key Terms at a Glance (continued)

- **Response prevention** — Blocking compulsive or avoidance responses during exposure-based work.
- **Restorative practice** — Voluntary processes addressing harm, accountability, and repair.
- **Risk assessment** — Structured identification and formulation of possible harms, contexts, and safeguards.
- **Risk factor** — Factor associated with increased probability of adverse outcome.
- **Role dispute** — IPT problem area involving non-reciprocal expectations in a relationship.
- **Role play** — Enactment of a situation for learning, rehearsal, or perspective taking.
- **Role transition** — IPT problem area involving change from an old role to a new one.
- **Routine outcome monitoring** — See outcome monitoring.
- **Rumination** — Repetitive, passive focus on distress and its causes or consequences.
- **Rupture (alliance)** — Strain or break in therapeutic collaboration, goals, tasks, or bond.
- **Safeguarding** — Protecting children and vulnerable people from abuse, neglect, exploitation, and harm.
- **Safety behaviour** — Action intended to prevent feared catastrophe that may maintain anxiety by blocking new learning.
- **Safety plan** — Collaborative plan identifying warning signs, coping, supports, professional help, and means safety.
- **Scaling question** — Solution-focused rating of present position, hope, confidence, or progress.
- **Schema** — Organised pattern of knowledge, belief, affect, and response.
- **Schema therapy** — Integrative therapy addressing early maladaptive schemas, coping styles, and modes.
- **School belonging** — Sense of acceptance, respect, safety, and participation at school.
- **School refusal** — Difficulty attending school associated with distress; requires functional and contextual assessment.
- **Screening** — Brief process identifying who may need fuller assessment.
- **Secondary emotion** — Emotion arising in response to a primary emotion, such as shame about anger.
- **Secure attachment** — Pattern in which care is sufficiently available and the child can use the caregiver as safe haven and secure base.
- **Secure base** — Relationship support from which exploration and learning occur.
- **Self-concept** — Organised perceptions and evaluations of oneself.

### Key Terms at a Glance (continued)

- **Self-determination** — Experience of autonomy, competence, and relatedness supporting motivation.
- **Self-efficacy** — Task- and context-specific belief in capability.
- **Self-esteem** — Global evaluation of self-worth.
- **Self-harm** — Intentional self-injury or poisoning irrespective of motive; intent and risk require assessment.
- **Self-monitoring** — Observing and recording one's behaviour, mood, thoughts, or physiology.
- **Self-regulation** — Managing attention, emotion, impulses, and behaviour toward goals and safety.
- **Self-soothing** — Strategies used to reduce distress; developmental capacity grows through co-regulation.
- **Sensorimotor** — Relating to sensation and movement.
- **Separation anxiety** — Excessive anxiety concerning separation from attachment figures or home.
- **Session rating scale** — Brief client feedback measure concerning session fit or alliance.
- **Setting event** — Broader contextual condition altering behaviour–consequence relations.
- **Sexual orientation** — Pattern of romantic, emotional, and/or sexual attraction.
- **Shame** — Painful self-conscious emotion involving perceived defectiveness or exposure.
- **Shaping** — Reinforcing successive approximations toward a target behaviour.
- **Sibling subsystem** — Family subsystem comprising siblings.
- **Significant other** — Important person in a client's relational world.
- **Single-case design** — Systematic evaluation of intervention effects with an individual over time.
- **Skill acquisition** — Learning through instruction, modelling, rehearsal, feedback, and practice.
- **Sleep hygiene** — Habits and environments supporting sleep; clinical sleep problems may require more than hygiene advice.
- **Social anxiety** — Fear of scrutiny, embarrassment, or negative evaluation in social or performance situations.
- **Social capital** — Resources available through relationships and networks.
- **Social clock** — Cultural expectations about timing of life transitions.
- **Social determinants of health** — Conditions in which people live, learn, work, and age that shape health.
- **Social learning** — Learning through observation, imitation, and vicarious consequences.

## Key Terms at a Glance (continued)

- **Social support** — Emotional, practical, informational, and companionship resources from relationships.
- **Solution-focused brief therapy** — Future-oriented approach using exceptions, scaling, goals, and client resources.
- **Somatic** — Relating to bodily experience.
- **Splitting** — Experiencing self or others in extreme all-good/all-bad terms; also team division into opposing views.
- **Stabilisation** — Phase of establishing safety, regulation, resources, and capacity before demanding processing.
- **Standardised measure** — Instrument administered and scored consistently with normative or criterion references.
- **Stepped care** — Matching intervention intensity to need and response, increasing or decreasing over time.
- **Stigma** — Social devaluation, stereotypes, prejudice, and discrimination linked to an identity or condition.
- **Stimulus** — Event or condition that may elicit or set the occasion for behaviour.
- **Strengths-based practice** — Assessment and intervention centred on capacities, resources, values, and hopes without denying harm.
- **Stress** — Response to demands perceived or experienced as exceeding resources.
- **Structural family therapy** — Minuchin's approach to subsystems, boundaries, hierarchy, and organisation.
- **Subjective units of distress (SUDS)** — Self-rating of distress, often 0–10 or 0–100.
- **Subsystem** — Part of a family system with its own roles and boundaries.
- **Suicidal ideation** — Thoughts about suicide, ranging from passive wishes to specific intent and plans; always requires contextual assessment.
- **Suicide risk formulation** — Dynamic explanation of factors, triggers, protective processes, and actions concerning suicide risk.
- **Supervision** — Professional process supporting competence, ethics, reflection, and accountability.
- **Support network** — People and organisations providing practical or emotional help.
- **Sustain talk** — Language favouring status quo or opposing a particular change.
- **Symptom** — Subjective experience or observable sign associated with distress or disorder.

## Key Terms at a Glance (continued)

- **Systemic** — Concerning interacting relationships, contexts, and feedback loops.
- **Teleological mode** — Mentalizing failure in which only visible action counts as proof of intention or care.
- **Teletherapy** — Counselling delivered through telecommunications.
- **Termination** — Ending phase or event in counselling.
- **Theory** — Organised set of concepts explaining phenomena and guiding observation or intervention.
- **Therapeutic alliance** — See alliance.
- **Therapeutic frame** — Boundaries of time, place, roles, fees where relevant, confidentiality, and contact.
- **Thought record** — Structured record of situation, thoughts, feelings, evidence, alternatives, and outcome.
- **Time-out** — Brief removal from reinforcing activity as part of a safe, planned behaviour strategy; not isolation or humiliation.
- **Titration** — Working in small doses to remain within tolerance.
- **Token economy** — System using tokens exchangeable for backup reinforcers.
- **Tolerance (window of)** — Arousal range in which a person can process, learn, and relate without overwhelm or shutdown.
- **Transactional analysis** — Berne's theory of ego states, transactions, games, scripts, and strokes.
- **Transference** — Relational expectations from earlier relationships activated in therapy.
- **Transition** — Change in role, setting, relationship, development, or status.
- **Trauma** — Experience or pattern that overwhelms coping and may produce lasting effects; definitions vary by framework.
- **Trauma-focused CBT** — Trauma-specific cognitive-behavioural treatment involving psychoeducation, regulation, narration/processing, and caregiver work.
- **Trauma-informed care** — Service approach recognising trauma prevalence and prioritising safety, trust, collaboration, empowerment, and cultural responsiveness.
- **Treatment fidelity** — See fidelity.
- **Treatment plan** — Document specifying formulation-based goals, interventions, responsibilities, measures, and review.
- **Triangle** — Three-person relational pattern in which tension between two is managed through a third.

## Key Terms at a Glance (continued)

- **Trigger** — Cue associated with emotional, bodily, behavioural, or memory responses.
- **Trust** — Expectation of reliability, honesty, benevolence, and respect.
- **Unconditional positive regard** — Rogers' non-possessive acceptance of the person.
- **Unconscious** — Mental processes outside current awareness.
- **Unique outcome** — Narrative term for events inconsistent with a problem-saturated story.
- **Universal prevention** — Intervention delivered to a whole population regardless of individual risk.
- **Validation** — Communicating that experience is understandable; not necessarily agreement with behaviour.
- **Validity** — Extent to which evidence supports interpretation and use of a measure or assessment.
- **Values** — Chosen directions or qualities guiding action.
- **Vicarious learning** — Learning by observing consequences experienced by others.
- **Vicarious trauma** — Practitioner changes resulting from empathic engagement with trauma material.
- **Visual schedule** — Pictorial or written sequence supporting predictability and independence.
- **Volition** — Capacity and experience of choosing or willing action.
- **Vulnerability** — Susceptibility to harm influenced by biology, development, relationships, and context.
- **Wellbeing** — Broad state of health, functioning, connection, meaning, and life satisfaction.
- **Withdrawal (social)** — Reduced participation in relationships or activities.
- **Withdrawal (substance)** — Physiological and psychological effects after reducing or stopping a substance.
- **Working alliance** — See alliance.
- **Working memory** — System for temporarily holding and manipulating information.
- **Wraparound care** — Coordinated, family-driven planning across services and natural supports.
- **Youth voice** — Meaningful participation of young people in decisions affecting them.
- **Zone of proximal development** — Vygotsky's range between independent performance and performance possible with support.

CHAPTER APPENDIX E

# Annotated Bibliography

Foundational works, contemporary reviews, and reading pathways.

This bibliography is a reading pathway, not a claim that every listed work is equally current or uncontested. Students should read primary texts alongside systematic reviews and critical literature.

## Foundations and Professional Practice

**Beck, J. S. (2020). *Cognitive Behavior Therapy: Basics and Beyond* (3rd ed.).** A clear clinical exposition of cognitive conceptualisation, session structure, and interventions. Best read as a treatment manual rather than a complete account of adolescent development.

**Carr, A. (2016). *The Handbook of Child and Adolescent Clinical Psychology* (3rd ed.).** A broad reference covering development, assessment, disorders, family contexts, and interventions. Useful for placing CAC. 701 theories in a wider clinical landscape.

**Corey, G. (2024). *Theory and Practice of Counseling and Psychotherapy* (11th ed.).** Accessible comparative introduction. Students should move from this survey to primary and evidence-focused sources.

**Johnstone, L., & Dallos, R. (Eds.). (2014). *Formulation in Psychology and Psychotherapy* (2nd ed.).** Cross-theoretical essays on formulation, power, narrative, and clinical judgement. Particularly valuable for Chapters 51–53.

**Kazdin, A. E. (2008). *Parent Management Training*.** A foundational account of behavioural parent training, treatment design, and evidence for disruptive behaviour.

**Norcross, J. C., & Goldfried, M. R. (Eds.). (2019). *Handbook of Psychotherapy Integration* (3rd ed.).** Comprehensive treatment of theoretical integration, eclecticism, common factors, and assimilative approaches. Use with Chapter 51.

**Prochaska, J. O., & Norcross, J. C. (2018). *Systems of Psychotherapy* (9th ed.).** Comparative theory text linking models to transtheoretical processes of change.

**Wampold, B. E., & Imel, Z. E. (2015). *The Great Psychotherapy Debate* (2nd ed.).** Major statement of common factors and contextual model. Read critically alongside disorder-specific treatment research.

**Weisz, J. R., & Kazdin, A. E. (Eds.). (2017). *Evidence-Based Psychotherapies for Children and Adolescents* (3rd ed.).** Essential evidence reference for youth treatments, moderators, implementation, and research limitations.

## Humanistic, Existential, Gestalt, and Narrative Works

**Cooper, M. (2008). *Essential Research Findings in Counselling and Psychotherapy*.** Helpful synthesis on relationship, client resources, and pluralistic practice.

**Epston, D., & White, M. (1992). *Experience, Contradiction, Narrative and Imagination*.** Selected papers showing narrative practice as conversation, documentation, and social action.

**Gendlin, E. T. (1978). *Focusing*.** Primary account of felt sense and experiential listening. Pair with contemporary evidence reviews rather than treating experiential report as outcome evidence.

**Mearns, D., Thorne, B., & McLeod, J. (2013). *Person-Centred Counselling in Action* (4th ed.).** Practical and relational account of Rogers' approach with attention to practitioner use of self.

**Perls, F., Hefferline, R., & Goodman, P. (1951). *Gestalt Therapy*.** Foundational but historically situated. Contemporary trauma and cultural critiques should inform use of experiments.

**Rogers, C. R. (1951). *Client-Centered Therapy*.** Primary source for the therapeutic conditions and client-directed change.

**Rogers, C. R. (1961). *On Becoming a Person*.** Accessible essays on self, relationships, education, and becoming. Useful for teaching but not a substitute for outcome research.

**White, M. (2007). *Maps of Narrative Practice*.** Mature exposition of externalising conversations, re-authoring, unique outcomes, and definitional ceremonies.

**White, M., & Epston, D. (1990). *Narrative Means to Therapeutic Ends*.** Foundational narrative text. Read with attention to power, discourse, and the social contexts of stories.

**Yalom, I. D. (1980). *Existential Psychotherapy*.** Classic organisation around death, freedom, isolation, and meaninglessness. Developmentally adapt before applying to younger adolescents.

## Cognitive and Third-Wave Works

**Beck, A. T. (1976). *Cognitive Therapy and the Emotional Disorders*.** Historical foundation of cognitive therapy and the cognitive model.

**Beck, A. T., Rush, A. J., Shaw, B. F., & Emery, G. (1979). *Cognitive Therapy of Depression*.** Primary treatment text whose session methods remain influential.

**Ellis, A. (1962). *Reason and Emotion in Psychotherapy*.** Foundational REBT statement. Students should compare its confrontational heritage with collaborative contemporary practice.

**Hayes, S. C., Strosahl, K. D., & Wilson, K. G. (2012). *Acceptance and Commitment Therapy* (2nd ed.).** Comprehensive ACT account including relational frame theory, psychological flexibility, values, and clinical methods.

**Kendall, P. C. (Ed.). (2012). *Child and Adolescent Therapy: Cognitive-Behavioral Procedures* (4th ed.).** Developmental CBT methods, anxiety programmes, family involvement, and school applications.

**Linehan, M. M. (2015). *DBT Skills Training Manual* (2nd ed.).** Core skills curriculum. Adolescent adaptations and family components should be studied before use.

**Miller, A. L., Rathus, J. H., & Linehan, M. M. (2007). *Dialectical Behavior Therapy with Suicidal Adolescents*.** Key adolescent DBT adaptation addressing self-harm, family involvement, and treatment structure.

**Segal, Z. V., Williams, J. M. G., & Teasdale, J. D. (2018). *Mindfulness-Based Cognitive Therapy for Depression* (2nd ed.).** Main MBCT text, including relapse prevention and inquiry. Adapt cautiously for adolescents and trauma presentations.

**Young, J. E., Klosko, J. S., & Weishaar, M. E. (2003). *Schema Therapy*.** Primary guide to schemas, coping styles, and modes. Useful where chronic interpersonal patterns persist.

## Behavioural and Learning Works

**Bandura, A. (1977). *Social Learning Theory*.** Foundational statement on modelling and vicarious learning.

**Bandura, A. (1997). *Self-Efficacy: The Exercise of Control*.** Comprehensive treatment of efficacy beliefs across development, health, education, and collective action.

**Cooper, J. O., Heron, T. E., & Heward, W. L. (2020). *Applied Behavior Analysis* (3rd ed.).** Detailed ABA reference. Read with disability ethics, assent, and historical critiques.

**Ferster, C. B., & Skinner, B. F. (1957). *Schedules of Reinforcement*.** Primary experimental source, historically important though not a clinical manual.

**Iwata, B. A., et al. (1994).** Research programme on functional analysis of self-injurious behaviour. Important for understanding experimental functional analysis and its constraints.

**Kazdin, A. E. (2005). *Parent Management Training*.** Evidence and procedures for changing coercive cycles and building positive parenting practices.

**Maag, J. W. (2018). *Behavior Management: From Theoretical Implications to Practical Applications* (3rd ed.).** School-oriented behavioural practice with functional and ethical dimensions.

**Martell, C. R., Dimidjian, S., & Herman-Dunn, R. (2022). *Behavioral Activation for Depression* (2nd ed.).** Clear behavioural activation manual applicable with developmental adaptation.

**Skinner, B. F. (1953). *Science and Human Behavior*.** Core radical behaviourist account. Pair with contemporary critiques and cognitive, cultural, and relational developments.

**Wolpe, J. (1958). *Psychotherapy by Reciprocal Inhibition*.** Historical source for systematic desensitisation and behavioural treatment of fear.

## Family, Attachment, and Interpersonal Works

**Ainsworth, M. D. S., Blehar, M. C., Waters, E., & Wall, S. (1978). *Patterns of Attachment*.** Foundational strange situation research. Read with cross-cultural critiques.

**Asen, E., & Fonagy, P. (2021). *Mentalization-Based Treatment with Families*.** Practical family MBT using curiosity, affect, perspective taking, and network meetings.

**Bowlby, J. (1988). *A Secure Base*.** Clinically accessible attachment essays on caregiving, loss, and therapeutic relationships.

**Goldenberg, I., Stanton, M., & Goldenberg, H. (2017). *Family Therapy: An Overview* (9th ed.).** Broad family therapy survey with models, techniques, and research.

**Haley, J. (1976). *Problem-Solving Therapy*.** Strategic family therapy text historically important for directives, hierarchy, and sequences.

**McGoldrick, M., Gerson, R., & Petry, S. (2008). *Genograms: Assessment and Intervention* (3rd ed.).** Practical guide to family diagrams and intergenerational patterns.

**Minuchin, S. (1974). *Families and Family Therapy*.** Foundational structural family therapy text.

**Mufson, L., Dorta, K. P., Moreau, D., & Weissman, M. M. (2011). *Interpersonal Psychotherapy for Depressed Adolescents* (2nd ed.).** Main IPT-A manual linking depression to grief, disputes, transitions, and deficits.

**Weissman, M. M., Markowitz, J. C., & Klerman, G. L. (2017). *The Guide to Interpersonal Psychotherapy* (updated ed.).** Concise IPT overview across applications.

## Trauma, Emotion, and Somatic Works

**Cohen, J. A., Mannarino, A. P., & Deblinger, E. (2017). *Treating Trauma and Traumatic Grief in Children and Adolescents* (2nd ed.).** Core TF-CBT text with caregiver components and developmental adaptations.

**Dana, D. (2018). *The Polyvagal Theory in Therapy*.** Clinically accessible polyvagal-informed exercises and language. Pair with scientific critiques of polyvagal claims.

**Greenberg, L. S. (2015). *Emotion-Focused Therapy: Coaching Clients to Work Through Their Feelings* (2nd ed.).** Updated EFT account of emotion schemes, markers, and tasks.

**Herman, J. L. (2015). *Trauma and Recovery* (2015 ed.).** Influential phase-oriented and political account of trauma, safety, remembrance, and reconnection.

**Ogden, P., & Fisher, J. (2015). *Sensorimotor Psychotherapy*.** Clinical methods for tracking sensation, posture, movement, and procedural patterns.

**Porges, S. W. (2011). *The Polyvagal Theory*.** Primary theoretical source. Students should also read critiques and current autonomic science.

**Shapiro, F. (2018). *Eye Movement Desensitization and Reprocessing (EMDR) Therapy* (3rd ed.).** Main EMDR text. Use certified training and child protocols before clinical delivery.

**van der Kolk, B. (2014). *The Body Keeps the Score*.** Influential synthesis of trauma, development, body, and treatment. Popular reach makes critical appraisal especially important.

## Child Development, Resilience, and Schools

**Blakemore, S.-J. (2018). *Inventing Ourselves*.** Accessible account of adolescent brain and social development.

**Bronfenbrenner, U. (1979). *The Ecology of Human Development*.** Primary ecological systems text.

**Dahl, R. E., Allen, N. B., Wilbrecht, L., & Suleiman, A. B. (2018).** "Importance of investing in adolescence from a developmental science perspective." *Nature, 554*, 441–450. A concise argument for adolescence as a sensitive developmental window.

**Masten, A. S. (2014). *Ordinary Magic: Resilience in Development*.** Comprehensive resilience synthesis emphasising ordinary adaptive systems.

**Steinberg, L. (2023). *Adolescence* (13th ed.).** Standard developmental text integrating biology, cognition, family, peers, school, and culture.

**Ungar, M. (2011).** "The social ecology of resilience." *American Journal of Orthopsychiatry, 81*(1), 1–17. Challenges individualistic resilience definitions.

**American School Counselor Association. (2022). *ASCA National Model* (4th ed.).** Comprehensive school counselling framework requiring local adaptation.

**Stone, C. B. (2022). *Ethics and Law for School Counselors* (5th ed.).** Detailed school confidentiality, records, and legal dilemmas.

## Culture, Feminist, Intersectional, and LGBTQ+ Practice

**American Psychological Association. (2015).** Guidelines for psychological practice with transgender and gender nonconforming people. Competencies for assessment, identity, stigma, and care systems.

**American Psychological Association. (2021).** Guidelines for psychological practice with sexual minority persons. Comprehensive affirmative practice resource.

**Crenshaw, K. (1989).** “Demarginalizing the intersection of race and sex.” Foundational intersectionality article grounded in Black feminist legal critique.

**Hook, J. N., et al. (2013).** Cultural humility research linking openness and alliance. Useful corrective to competence as static mastery.

**Meyer, I. H. (2003).** Minority stress model for lesbian, gay, and bisexual populations. Extend cautiously and intersectionally to other groups.

**Ratts, M. J., et al. (2016).** Multicultural and Social Justice Counseling Competencies. Framework linking self-awareness, client worldview, relationship, and advocacy.

**Ryan, C., et al. (2009).** Family rejection and health outcomes research. Pair with Family Acceptance Project practice materials.

**Sue, D. W., Sue, D., Neville, H. A., & Smith, L. (2022). *Counseling the Culturally Diverse* (9th ed.).** Major multicultural counselling text; read with attention to evolving terminology and evidence.

**Worell, J., & Remer, P. (2003). *Feminist Perspectives in Therapy*.** Empowerment model integrating gender, power, and social change.

## Motivational, Assessment, and Outcome Works

**Hunsley, J., & Mash, E. J. (Eds.). (2018). *A Guide to Assessments that Work* (2nd ed.).** Evidence-focused assessment across disorders and populations.

**Lambert, M. J. (2010). *Prevention of Treatment Failure*.** Outcome feedback and deterioration detection.

**Miller, W. R., & Rollnick, S. (2023). *Motivational Interviewing* (4th ed.).** Current comprehensive MI text.

**Naar, S., & Suarez, M. (2021). *Motivational Interviewing with Adolescents and Young Adults* (2nd ed.).** Developmental, family, and systems adaptations.

**Rollnick, S., Kaplan, S. G., & Rutschman, R. (2016). *Motivational Interviewing in Schools*.** Brief conversations for attendance, behaviour, learning, and health.

## Research and Teaching Works

**Ambrose, S. A., et al. (2010). *How Learning Works*.** Seven research-based principles for teaching.

**Biggs, J., & Tang, C. (2011). *Teaching for Quality Learning at University* (4th ed.).** Constructive alignment and assessment design.

**Brookfield, S. D. (2017). *Becoming a Critically Reflective Teacher* (2nd ed.).** Power, student perception, and reflective teaching.

**Hattie, J., & Timperley, H. (2007).** “The power of feedback.” *Review of Educational Research, 77*(1), 81–112. Distinguishes feedback about task, process, self-regulation, and self.

**Weisz, J. R., et al. (2017).** Youth psychotherapy evidence syntheses and meta-analyses. Important for understanding modest average effects, heterogeneity, and implementation.

## Chapter-by-Chapter Study Notes

The following notes help students connect the bibliography to seminar work. For each chapter, identify one primary source, one evidence source, and one critical or youth-specific source. Do not treat a classic text as current evidence merely because it is historically important.

**Chapter 1: The Role of Theory in Professional Counseling Practice.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 2: Scientific and Professional Foundations of Counseling.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 3: Philosophical Worldviews: The Five Domains of Theory.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 4: A Genealogy of Counseling Theories.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 5: The Therapeutic Relationship: Alliance, Rupture, and Repair.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source

that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 6: Common Factors, Placebo Effects, and Specific Factors.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 7: The Evidence Base: Evaluating Counseling Theories.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 8: Culturally Responsive Theory and the Cultural Lens.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 9: Developmental Foundations for Counseling Youth.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 10: Person-Centered Theory: The Client as Expert.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 11: Existential Therapy: Meaning, Isolation, and Freedom.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar,

students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 12: Gestalt Therapy: Awareness, Contact, and the Here and Now.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 13: Focusing-Oriented Therapy and the Felt Sense.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 14: Transactional Analysis: Structure, Strategy, and Script.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 15: Internal Family Systems and Structural Dissociation.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 16: Cognitive Therapy: The Cognitive Model.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 17: Cognitive Therapy with Children and Adolescents.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and

one decision they would monitor locally if using the approach.

**Chapter 18: Rational Emotive Behavior Therapy.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 19: Schema Therapy: Modes, Styles, and Early Maladaptive Schemas.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 20: Acceptance and Commitment Therapy.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 21: Dialectical Behavior Therapy and Its Adolescent Adaptations.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 22: Mindfulness-Based Cognitive Therapy and Mindfulness Approaches.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 23: Behavior Therapy: Principles of Learning.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 24: Applied Behavior Analysis: Functional Assessment.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 25: Exposure, Modeling, and Behavioral Skills Training.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 26: Operant Conditioning and Contingency Management.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 27: Behavioral Activation and Activity Scheduling.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 28: Contingency Management in Schools and Clinics.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 29: Functional Analysis of Self-Injurious and Aggressive Behaviour.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 30: Family Systems Theory: Bowen and the Generational Project.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 31: Structural Family Therapy.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 32: Strategic Family Therapy and Communication Theory.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 33: Family Therapy with Parents and Adolescents.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 34: Attachment Theory and Attachment-Based Therapy.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 35: Solution-Focused Brief Therapy and Strength-Based Practice.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 36: Narrative Therapy and the Re-Authoring of Lives.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 37: Emotion-Focused Therapy.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 38: Trauma-Informed Practice: Principles, Phases, and Precautions.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 39: EMDR and Bilateral Stimulation.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 40: Stabilisation, Grounding, and Phase-Oriented Work.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 41: Interpersonal Psychotherapy for Adolescents.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 42: Mentalization-Based Treatment.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 43: Somatic, Sensorimotor, and Polyvagal-Informed Practice.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 44: Motivational Interviewing.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 45: Social Cognitive Learning Theory.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 46: Adolescent Development: Theory and Counseling Implications.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 47: Psychodynamic and Object Relations Approaches.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 48: Multicultural, Feminist, and Intersectional Theory.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 49: Affirmative Practice with LGBTQ+ Youth.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 50: Resilience, Ecological Systems, and Strengths-Based Practice.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 51: Theoretical Integration and the Integrative Stance.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 52: Case Conceptualization and Treatment Planning.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 53: Assessment and Outcome Monitoring in Counseling.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 54: Termination, Evaluation, and the Ethics of Endings.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 55: Teletherapy and Technology in Counseling with Youth.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 56: School Counseling Practice and Consultation.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 57: Teaching CAC. 701: Session Plans and Facilitation.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

**Chapter 58: Assessment Blueprint, Assignments, and Rubrics.** Begin with the chapter's primary theorists and then locate a systematic review, guideline, or well-designed outcome study relevant to adolescents. Ask what population was studied, what comparison was used, which outcomes mattered to young people and families, and what implementation resources were assumed. Finish by finding one source that criticises the model's cultural assumptions, mechanism, or transfer from adults to youth. In seminar, students should be able to state one claim supported by the literature, one claim that remains contested, and one decision they would monitor locally if using the approach.

CHAPTER APPENDIX F

# Session and Worksheet Templates

Reusable session outlines, thought records, scales, and trackers.

## F1. Initial Session Contracting Worksheet

Young person's preferred name and safe contact method: _____

Who referred me and what I understand the reason to be: _____

What I want to be different: _____

What adults want to be different: _____

What counselling can and cannot do here: _____

What will stay private and what may need to be shared: _____

People I want involved or not involved: _____

How I can pause, stop, ask questions, or change activity: _____

How appointments, cancellations, messages, and emergencies work: ____

How we will know whether counselling is helping: _____

First review date: _____

## F2. Session Planning and Review Sheet

Session date/number: _____ Present: _____

Opening check-in and current state: _____

Young person's agenda: _____

Risk or safeguarding changes: _____

Goal focus for today: _____

Intervention and theory rationale: _____

What the young person did, said, or discovered: _____

What changed in the room: _____

Family/school/system action agreed: _____

Practice before next session: _____

Rating of usefulness (0–10) and one-point-higher change: _____

Plan for next session: _____

## F3. Five-Part Formulation Worksheet

**Presenting pattern:** When, where, with whom, how often, impact?

**Predisposing factors:** Development, temperament, health, learning, relationships, culture, prior adversity and coping.

**Precipitating factors:** Recent changes, losses, transitions, demands, conflicts.

**Perpetuating factors:** Avoidance, reinforcement, beliefs, arousal, family/school sequences, accommodation, insomnia, practical barriers, discrimination, service fragmentation.

**Protective factors:** Relationships, skills, interests, school/community resources, values, prior successes, practical assets.

**One-sentence collaborative formulation:** “When _____ happens, I tend to _____ because _____; this helps by _____ but keeps _____ going. I already have _____ that we can build on. First we will _____.”

**What would show this formulation is wrong or incomplete?** _____

## F4. ABC and Function Worksheet

Behaviour in observable words: _____

| Date/time/setting | Antecedent | Behaviour | Immediate consequence | Longer consequence |
|---|---|---|---|---|
| | | | | |
| | | | | |
| | | | | |

What may the behaviour obtain, avoid, communicate, regulate, or protect?

Possible functions to test: attention / escape / tangible or activity / sensory or automatic / communication / safety / status / other.

Safer or more effective replacement behaviour: _____

Who must change responses, and how? _____

Data to collect and review date: _____

## F5. Exposure and Mastery Ladder

Feared or avoided situation: _____

Belief/prediction to test: _____

Safety facts and genuine risks checked: _____

| Step | Situation | Predicted distress 0–10 | Actual distress | What happened? | What did I learn? |
|---|---|---|---|---|---|
| 1 | | | | | |
| 2 | | | | | |
| 3 | | | | | |
| 4 | | | | | |
| 5 | | | | | |

Support person and role: _____

Safety behaviours to reduce gradually: _____

Stopping/safety conditions: _____

Repeat and generalisation plan: _____

## F6. Behavioural Activation Weekly Planner

Activities to restore: pleasure / mastery / social / movement / rest / learning / contribution.

| Day | Time | Activity | With whom | Predicted pleasure 0–10 | Predicted mastery 0–10 | Actual pleasure | Actual mastery | Notes |
|---|---|---|---|---|---|---|---|---|
| | | | | | | | | |
| | | | | | | | | |
| | | | | | | | | |

Smallest possible first step: _____

Obstacle most likely to appear: _____

If–then plan: If _____, then I will _____.

Adult or peer support: _____

## F7. Communication Chain Worksheet

Situation: _____

What the other person said/did (observable): _____

What I thought it meant: _____

Feeling(s) and body sensations: _____

What I wanted or needed: _____

What I said/did, including tone and timing: _____

What the other person may have thought it meant: _____

What happened next: _____

What I want them to understand: _____

One sentence I could try: _____

Support, timing, place, and pause plan: _____

## F8. Problem-Solving Worksheet

Problem in specific words: _____

What I can influence / cannot influence / need help with: _____

Goal for a good-enough outcome: _____

Options without judging them yet:

1. _____
2. _____
3. _____
4. _____

For the two most promising options, list likely benefits, costs, risks, and who is affected.

Option chosen and first small step: _____

Who needs to know or help? _____

When will I review what happened? _____

What I learned even if it did not work: _____

## F9. Coping and Grounding Menu

My early warning signs in body, thoughts, feelings, and behaviour:

_____

Things that help when arousal is rising:

- Movement: _____
- Sensory grounding: _____
- Breathing or voice: _____
- Place or object: _____
- Person to contact: _____
- Words that help me: _____

Things that help when I shut down or feel blank:

_____

Things that usually make it worse: _____

What adults can do that helps: _____

What adults should avoid: _____

When to seek urgent help: _____

## F10. Personal Safety Plan

This plan supplements, and does not replace, professional risk assessment and local crisis procedures.

Warning signs that a crisis may be developing: _____

Internal coping strategies I can try without harming myself: _____

People or places that provide distraction or support: _____

People I can ask for help: _____

Professionals/services and contact details: _____

Steps to make the environment safer, including limiting access to means discussed with a responsible professional: _____

What adults should do if they are worried: _____

What I want adults not to do unless safety requires it: _____

Reasons for living, people, hopes, or commitments that matter to me: _

Plan created with: _____ Date: _____ Review: _____

## F11. Family Meeting Preparation Sheet

Purpose of meeting in one sentence: _____

Young person's three priorities:

1. _____
2. _____
3. _____

Parent/carer priorities: _____

School or professional priorities, if relevant: _____

Topics not to discuss in joint session and why: _____

One recent example to examine slowly: _____

Requests each person wants to make: _____

Agreements we hope to leave with: _____

Who will write and share the summary? _____

Review date: _____

## F12. Goal Review and Ending Worksheet

Original goals: _____

What has changed in symptoms, functioning, relationships, school participation, and daily life? _____

Which changes matter most to me? _____

What helped most? _____

What did not help or felt harmful? _____

Skills and supports I will keep using: _____

Early warning signs of setback: _____

First three steps if difficulties return: _____

People and services I can contact: _____

What remains unfinished and where it can be addressed: _____

Do I want a booster, transfer, or no further counselling now? _____

Closing summary agreed and copies: _____

CHAPTER APPENDIX G

# Supervision and Reflection Templates

Structured instruments for supervision, consultation, and reflective practice.

## G1. Purpose of Supervision

Supervision protects clients, develops practitioners, and sustains ethical judgement. It is not case gossip, managerial surveillance alone, or a place to obtain permission for decisions the practitioner remains responsible for making. Good supervision integrates clinical formulation, intervention skills, relationship processes, culture and power, risk, organisational context, and practitioner wellbeing.

Students should prepare cases before supervision and identify the decision or uncertainty they are bringing. “Tell me what to do” is less useful than “I am caught between protecting confidentiality and involving a parent; here are the facts, risks, options, and my current leaning.”

## G2. Case Presentation Template

Client identifier and age band: _____

Service context and mandate: _____

Referral question and who defined it: _____

Young person's goals in their words: _____

Presenting pattern and settings: _____

Relevant development, health, learning, culture, family, peers, school, digital and material context: _____

Strengths and resources: _____

Risk, safeguarding, and protective factors: _____

Current formulation (five-part if useful): _____

Theory/interventions used and response: _____

Outcome data and client feedback: _____

Relationship processes, ruptures, practitioner reactions: _____

Ethical, cultural, legal, or organisational tensions: _____

Specific supervision question: _____

Options considered and provisional decision: _____

Actions agreed, owner, and date: _____

## G3. Reflective Practice Prompts

After a session, consider:

- What did I notice first, and what did I not notice?
- Whose language dominated my formulation—the young person's, parent's, school's, theory's, or mine?
- What did I feel in my body, and how did it influence pace or intervention?
- Where did I rush to fix, rescue, withdraw, confront, or please?
- What cultural or power assumption may have shaped my reading?

- What did the young person teach me about what helps?
- What would I do differently with the same information?
- What do I need to learn, read, practise, or refer?
- Is there a record, safeguarding, consent, or follow-up action outstanding?

Reflection is incomplete if it produces only self-criticism. It should produce a testable change in attention, behaviour, or systems.

## G4. Alliance Rupture Review

Session/date: _____

What was the rupture signal—withdrawal, confrontation, missed session, silence, compliance without engagement, sudden deterioration?

What happened immediately before it?

What might the young person have experienced?

What was my contribution?

What repair was attempted and how did it land?

What confidentiality, family, cultural, or institutional factor was involved?

What will I do at the next contact?

What supervision or team action is needed?

## G5. Risk Review Template

Current concern and time frame: _____

Static/history factors: _____

Dynamic factors and triggers: _____

Warning signs and likely scenarios: _____

Protective factors and reasons for living/engaging: _____

Young person's view and preferred help: _____

Family/school/service safeguards: _____

Actions taken, by whom, and when: _____

Information shared and basis: _____

Review trigger and date: _____

Risk formulation should be updated when circumstances change. Avoid copying forward “low risk” without fresh evidence.

## G6. Practitioner Wellbeing and Caseload Review

Current caseload complexity: _____

Cases involving death, trauma, conflict, or moral distress: _____

Sleep, workload, supervision access, and recovery time: _____

Signs of compassion fatigue, secondary traumatic stress, cynicism, over-involvement, or avoidance: _____

Supports and boundaries to strengthen: _____

Organisational changes to request: _____

Learning or referral decision arising from limits: _____

Wellbeing work is not individual resilience training alone. Unsafe staffing, impossible caseloads, poor supervision, and discriminatory workplaces require organisational action.

## G7. Individual Supervision Agenda

1. Immediate safety, safeguarding, legal, or ethical issues.
2. Cases with deterioration, rupture, or uncertainty.
3. One case presented in depth.
4. Skills observation or transcript review.
5. Cultural, power, and practitioner-position reflection.
6. Service constraints and advocacy.
7. Learning actions and reading.
8. Practitioner wellbeing and boundaries.
9. Review of previous actions.

Supervision record: date, attendees, cases by identifier, decisions, rationale, actions, review dates. Store supervision records according to policy, separate from client notes where required.

## G8. Group Supervision Protocol

A 90-minute group might include:

- five minutes: urgent risk or ethical notices;
- twenty minutes: Case A presentation and clarifying questions;
- fifteen minutes: multiple theoretical readings;
- ten minutes: agreed actions;
- twenty minutes: Case B skills transcript or recording (with consent);
- fifteen minutes: systems and equity theme;
- five minutes: learning commitments.

Ground rules: confidentiality within lawful limits, no identifiable information beyond need, curiosity before advice, challenge ideas not people, no requirement to disclose personal trauma, and explicit attention to whose perspective is missing.

## G9. Peer Observation Feedback Form

Observed capability and context: _____

Consent and contracting observed: _____

Strengths with behavioural evidence:

1. _____
2. _____
3. _____

Moments where practice was effective: _____

Moment requiring development and exact alternative wording/action: ____

Developmental and cultural responsiveness: _____

Safety, boundaries, and ethics: _____

One priority for next practice: _____

Observer signature/date: _____ Practitioner response: _____

## G10. Reflective Essay Structure

Use this structure for assessed reflection without disclosing unnecessary personal or client material.

**Situation.** Briefly describe the event using fictional or properly anonymised material.

**Observation.** Separate facts, feelings, bodily responses, and interpretations.

**Theory.** Apply at least two concepts from the course and identify where they conflict.

**Culture and power.** Examine identity, institutional position, language, and whose knowledge was privileged.

**Ethics.** Identify duties, consent, confidentiality, safeguarding, and boundaries.

**Learning.** State what changed in your understanding, with evidence.

**Action.** Specify one practice change, one learning activity, and one supervision question.

**Evaluation.** How will you know whether the change helped a future client?

## G11. Monthly Practitioner Development Log

| Date | Capability practised | Source/supervision | Client or simulated context | Evidence of change | Next step |
|---|---|---|---|---|---|
| | | | | | |
| | | | | | |
| | | | | | |

Competence should be claimed narrowly. “Completed a workshop” is exposure; “delivered under supervision with fidelity and reviewed outcomes” is stronger evidence.

## G12. Difficult Supervision Questions

- What am I avoiding asking this client or family?
- Which theory am I using to protect myself from uncertainty?
- Who would describe this case completely differently?
- What if the problem is the service arrangement, not the young person?
- Am I continuing because it helps, because ending feels like rejection, or because the system rewards activity?
- What would I do if this were my last session?
- What evidence would make me refer, escalate, or stop?
- Whose voice has not been heard directly?
- What is the smallest ethical action available today?

## Chapter-Linked Supervision Prompts

Select the prompt corresponding to the model being used. Bring evidence from a session, record, measure, or observed interaction rather than only a general impression.

**Chapter 1 — The Role of Theory in Professional Counseling Practice:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 2 — Scientific and Professional Foundations of Counseling:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 3 — Philosophical Worldviews: The Five Domains of Theory:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 4 — A Genealogy of Counseling Theories:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 5 — The Therapeutic Relationship: Alliance, Rupture, and Repair:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 6 — Common Factors, Placebo Effects, and Specific Factors:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 7 — The Evidence Base: Evaluating Counseling Theories:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 8 — Culturally Responsive Theory and the Cultural Lens:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 9 — Developmental Foundations for Counseling Youth:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 10 — Person-Centered Theory: The Client as Expert:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 11 — Existential Therapy: Meaning, Isolation, and Freedom:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 12 — Gestalt Therapy: Awareness, Contact, and the Here and Now:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 13 — Focusing-Oriented Therapy and the Felt Sense:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 14 — Transactional Analysis: Structure, Strategy, and Script:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 15 — Internal Family Systems and Structural Dissociation:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 16 — Cognitive Therapy: The Cognitive Model:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 17 — Cognitive Therapy with Children and Adolescents:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 18 — Rational Emotive Behavior Therapy:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 19 — Schema Therapy: Modes, Styles, and Early Maladaptive Schemas:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 20 — Acceptance and Commitment Therapy:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 21 — Dialectical Behavior Therapy and Its Adolescent Adaptations:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 22 — Mindfulness-Based Cognitive Therapy and Mindfulness Approaches:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 23 — Behavior Therapy: Principles of Learning:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 24 — Applied Behavior Analysis: Functional Assessment:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 25 — Exposure, Modeling, and Behavioral Skills Training:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 26 — Operant Conditioning and Contingency Management:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 27 — Behavioral Activation and Activity Scheduling:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 28 — Contingency Management in Schools and Clinics:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 29 — Functional Analysis of Self-Injurious and Aggressive Behaviour:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 30 — Family Systems Theory: Bowen and the Generational Project:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 31 — Structural Family Therapy:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 32 — Strategic Family Therapy and Communication Theory:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 33 — Family Therapy with Parents and Adolescents:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 34 — Attachment Theory and Attachment-Based Therapy:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 35 — Solution-Focused Brief Therapy and Strength-Based Practice:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 36 — Narrative Therapy and the Re-Authoring of Lives:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 37 — Emotion-Focused Therapy:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 38 — Trauma-Informed Practice: Principles, Phases, and Precautions:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 39 — EMDR and Bilateral Stimulation:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 40 — Stabilisation, Grounding, and Phase-Oriented Work:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 41 — Interpersonal Psychotherapy for Adolescents:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 42 — Mentalization-Based Treatment:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 43 — Somatic, Sensorimotor, and Polyvagal-Informed Practice:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 44 — Motivational Interviewing:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 45 — Social Cognitive Learning Theory:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 46 — Adolescent Development: Theory and Counseling Implications:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 47 — Psychodynamic and Object Relations Approaches:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 48 — Multicultural, Feminist, and Intersectional Theory:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 49 — Affirmative Practice with LGBTQ+ Youth:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 50 — Resilience, Ecological Systems, and Strengths-Based Practice:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 51 — Theoretical Integration and the Integrative Stance:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 52 — Case Conceptualization and Treatment Planning:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 53 — Assessment and Outcome Monitoring in Counseling:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 54 — Termination, Evaluation, and the Ethics of Endings:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 55 — Teletherapy and Technology in Counseling with Youth:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 56 — School Counseling Practice and Consultation:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 57 — Teaching CAC. 701: Session Plans and Facilitation:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

**Chapter 58 — Assessment Blueprint, Assignments, and Rubrics:** What did the young person understand the purpose of the work to be? Which concept from this approach most influenced your intervention, and what exactly did you do differently because of it? What evidence supports the formulation, what evidence is missing, and what alternative explanation remains plausible? Identify the developmental, cultural, family, school, and safeguarding implications. State one moment where your own reaction may have altered the session, one limit of competence, and the specific outcome you will review next time.

CHAPTER APPENDIX H

# Referral, Documentation, and Crisis Resources

Referral mapping, documentation standards, and safety planning.

# H1. Principles for Referral

Referral is a clinical intervention, not disposal. A good referral matches need, urgency, competence, setting, culture and language, disability access, family logistics, and young person's preference. It includes a specific question, relevant information, consent or lawful basis, risk communication, and follow-up to confirm receipt.

Refer when needs exceed practitioner competence or scope, when medical or psychiatric assessment is required, when risk cannot be managed in the setting, when specialist treatment is indicated, when conflict of interest arises, or when the young person requests another service and it is clinically appropriate.

Do not refer merely because a case is complex, unrewarding, culturally unfamiliar, or likely to affect outcome statistics. Complexity may require consultation and joint work rather than transfer.

# H2. Referral Decision Checklist

- What decision or treatment can the receiving service provide that this service cannot?
- How urgent is it, and what must happen while waiting?
- Has medical assessment been considered?
- What safeguarding route applies?
- What does the young person want, fear, or expect from referral?
- What information is necessary, and what consent or lawful basis supports sharing?
- Which language, communication, disability, transport, cost, and digital access arrangements are needed?
- Who owns follow-up if the referral is rejected, delayed, or lost?
- How will current support continue during transition?
- What feedback loop will confirm attendance, assessment, and plan?

# H3. Common Referral Pathways

**Primary care/medical:** unexplained physical symptoms, pain, sleep disorders, eating and growth concerns, medication questions, puberty concerns, chronic illness, injury, sexual health, substance effects.

**Child and adolescent mental health services:** moderate to severe mental health difficulties, diagnostic complexity, self-harm, trauma symptoms, eating disorders, psychosis, severe anxiety or depression, neurodevelopmental assessment pathways.

**Emergency/crisis services:** immediate risk to life, serious injury, acute intoxication, psychosis with danger, inability to maintain safety. Use local emergency numbers and procedures.

**Safeguarding/children's social care:** suspected abuse, neglect, exploitation, trafficking, domestic abuse, unsafe care arrangements, missing episodes, serious welfare concerns.

**Education services:** special educational needs, disability accommodations, attendance, exclusion prevention, educational psychology, alternative provision, speech and language, occupational therapy.

**Substance services:** harmful use, dependence, withdrawal risk, drug-related exploitation, family support.

**Sexual health and exploitation services:** contraception, STI testing, pregnancy options under local law, sexual assault care, online exploitation, healthy relationships.

**Housing and family support:** homelessness, eviction, young caring, benefits, food insecurity, domestic abuse support, respite.

**Youth and community services:** mentoring, sport, arts, culturally specific support, LGBTQ+ youth support, disability groups, bereavement support, young carer services.

Local names, thresholds, and contact details must be inserted by each service and reviewed regularly.

## H4. Warm Handover Script

"I want to explain why I am suggesting this referral and what will happen next. Your counselling here does not have to end abruptly unless that is the service rule, and I will tell you if it does. The specialist service can help with _____. I recommend it because _____. Alternatives include _____. The referral will say _____ and will not include _____, except for safety information I have explained. You can tell me if any wording is factually wrong. I will check on _____ date whether they received it. While we wait, our plan is _____. If things become unsafe, contact _____."

Where the young person disagrees, document their view, assess capacity and risk, explain any duty to proceed, and preserve choices over timing, advocate, language, and information where possible.

## H5. Crisis Resource Card Template

Services should replace bracketed fields with verified local information and review the card at least annually.

**If there is immediate danger to life:** [local emergency number]

**24-hour crisis line:** [name, number, text/web options]

**Child protection/safeguarding:** [local route]

**Sexual assault support:** [service]

**Domestic abuse support:** [service]

**Substance emergency/poison information:** [service]

**LGBTQ+ youth support:** [service]

**Bereavement support:** [service]

**Housing emergency:** [service]

**My trusted adult:** _____ Phone: _____

**My counsellor/service and hours:** _____

**My safety plan is located:** _____

A crisis card is useful only if the young person can access it privately and believes the services will respond. Check phone credit, shared-device privacy, language, disability access, and whether contacting a service could alert an unsafe person.

## H6. Documentation Standards

Documentation should be timely, factual, proportionate, and respectful. Separate observation, report, inference, decision, and action. Record consent, information sharing, risk formulation, and review dates. Avoid jargon and moral labels.

Useful records include:

- referral and consent;
- assessment and formulation;
- treatment plan;
- session summaries;
- risk and safety plans;
- parent, school, and inter-agency contacts;
- measures and reviews;
- incidents and safeguarding actions;
- endings, referrals, and closing summaries.

Do not record speculation as fact, copy old conclusions without review, or include unnecessary information about third parties. Remember that young people and families may later read records; respectful writing is both an ethical and relational practice.

## H7. Immediate Crisis Response Sequence

This sequence supports professional judgement and local emergency procedures; it does not replace them.

1. **Ensure immediate safety.** Reduce access to means where possible, involve responsible adults or emergency services, and do not leave a young person alone if policy and risk require continuous support.
2. **Use a calm, direct approach.** Ask clearly about thoughts, intent, plans, means, intoxication, injuries, and immediate supports.
3. **Obtain medical help** for injury, ingestion, overdose, severe agitation, psychosis, or physical deterioration.
4. **Contact safeguarding and emergency supports** according to law and policy.
5. **Limit information sharing** to what is needed for protection, while being honest with the young person where safe.
6. **Document** times, observations, statements, actions, consultations, and handovers.
7. **Arrange follow-up** before the young person leaves the practitioner's responsibility. A referral sent is not a risk managed.
8. **Support staff and family**, review the response, and update the plan.

Do not ask a young person to promise never to self-harm as a substitute for assessment and safety planning. “No-suicide contracts” are not evidence-based protections.

## H8. Post-Crisis Review

Within an appropriate period after a crisis, review:

- What happened, in sequence?
- What warning signs were present or missed?
- What helped and what increased distress?
- Were confidentiality, dignity, culture, disability, and language handled well?
- Were family, school, health, and safeguarding communications timely?
- Was any restrictive intervention used, and was it lawful, proportionate, documented, and reviewed?
- What needs to change in the safety plan, environment, staffing, or referral route?
- Does the young person want a debrief, repair conversation, advocate, or complaint route?
- What support do involved staff need?

Post-crisis learning should improve systems without converting a frightened young person into a case study without consent.

## H9. Service Directory Worksheet

| Need | Local service | Eligibility | Contact | Referral route | Waiting time | Access/language notes | Verified date |
|---|---|---|---|---|---|---|---|
| Crisis | | | | | | | |
| Mental health | | | | | | | |
| Safeguarding | | | | | | | |
| Medical | | | | | | | |
| Eating concerns | | | | | | | |
| Substance use | | | | | | | |
| Sexual health/assault | | | | | | | |
| Housing | | | | | | | |
| Disability/SEND | | | | | | | |
| LGBTQ+ youth | | | | | | | |
| Young carers | | | | | | | |

| Need | Local service | Eligibility | Contact | Referral route | Waiting time | Access/language notes | Verified date |
|---|---|---|---|---|---|---|---|
| Bereavement | | | | | | | |

Review the directory at least every six months and after any failed referral.

## H10. Referral Tracking Log

Client identifier: _____ Referred to: _____

Date sent: _____ Sent by: _____ Consent/basis: _____

Acknowledged date: _____ Appointment offered: _____

Attended/declined/not eligible: _____

Outcome or plan received: _____

Current service continuing support: _____

Follow-up owner and next check date: _____

Reason for rejection and alternative plan: _____

## H11. Inter-Agency Meeting Record

Purpose and decisions required: _____

Attendees and roles: _____

Young person's views and how represented: _____

Family views: _____

Agreed formulation or shared facts: _____

Disagreements and uncertainties: _____

Actions, owners, dates: _____

Information-sharing agreements: _____

Risk/safety arrangements: _____

Review date: _____

## H12. Practitioner Crisis Pack Checklist

A service should maintain current, accessible information on:

- emergency and safeguarding contacts;
- designated safeguarding leads;
- local mental health crisis pathways;
- medical and sexual assault services;

- interpreter access;
- out-of-hours responsibilities;
- incident reporting;
- lone-working rules;
- remote-session emergency procedures;
- post-incident supervision and debrief;
- complaint and advocacy routes.

Practitioners should not carry the entire system in personal memory. Crisis competence is partly organisational design.

## Presentation-Specific Referral Pathways

Pathways must be localised. A referral should state the decision requested and continue support while waiting.

### Persistent school avoidance

Potential services include school leadership, attendance, health, educational psychology, mental health, family support. The referral should include attendance pattern, safety, curriculum access, health and sleep, family plan, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

### Self-harm or suicidal concern

Potential services include crisis, primary care, mental health, safeguarding where indicated. The referral should include current intent, means, injuries, protective factors, follow-up owner, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

### Suspected exploitation

Potential services include safeguarding, police or specialist services under local procedure, health, youth support. The referral should include immediate safety, locations, online evidence handled by specialists, trusted adult, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Eating or weight concern

Potential services include primary care/paediatrics, dietetics, eating disorder service, mental health. The referral should include growth, medical observations through clinicians, food pattern, exercise, family stress, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Substance-related harm

Potential services include substance service, primary care, safeguarding, youth work. The referral should include substances, frequency, withdrawal, supply, exploitation, motivation, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Possible neurodevelopmental need

Potential services include paediatrics, educational psychology, speech/language, occupational therapy, school SEND. The referral should include developmental history, settings, communication, sensory, learning profile, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Bereavement with functional decline

Potential services include school support, bereavement service, primary care, mental health if indicated. The referral should include relationship, rituals, functioning, trauma symptoms, family support, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Family conflict or unsafe discipline

Potential services include family support, safeguarding, mediation where safe, mental health. The referral should include violence/coercion, child voice, separate accounts, immediate safety, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person

disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Sexual health or assault concern

Potential services include sexual health, sexual assault service, safeguarding, primary care. The referral should include consent, timing, medical needs, forensic guidance by specialists, support person, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Housing or food insecurity

Potential services include housing, benefits/family support, school, community resources. The referral should include immediate accommodation, food, transport, school continuity, confidentiality, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Gender or sexuality-related distress and rejection

Potential services include affirming youth support, school safeguarding, family support, health pathways as requested. The referral should include chosen language, outness by context, bullying, home safety, support network, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

## Chronic pain or unexplained physical symptoms

Potential services include primary care/paediatrics, school health, physiotherapy or relevant specialists, mental health in parallel. The referral should include medical assessment, functioning, sleep, school participation, symptom pattern, plus the young person's goals, communication and access needs, relevant strengths, current supports, and what has already been tried. State urgency honestly and identify who will follow up if the referral is delayed or rejected. Avoid copying entire therapy notes; share the minimum necessary for the decision. Where the young person disagrees with referral, record their view and explain any duty to proceed. A pathway is not complete until receipt is confirmed and the young person knows what happens next.

CHAPTER APPENDIX I

# Glossary of Acronyms and Course FAQ

Abbreviations used throughout the book and answers to recurring student questions.

# I1. Acronyms and Abbreviations

| Acronym | Meaning |
|---|---|
| ABA | Applied behaviour analysis |
| ABC | Antecedent–Behaviour–Consequence; also Attachment and Biobehavioral Catch-up in attachment literature |
| ACEs | Adverse childhood experiences |
| ACT | Acceptance and commitment therapy |
| ADHD | Attention-deficit/hyperactivity disorder |
| APA | American Psychological Association |
| ASD | Autism spectrum disorder (terminology varies by context) |
| BACP | British Association for Counselling and Psychotherapy |
| CBT | Cognitive-behavioural therapy |
| CFTSI | Child and Family Traumatic Stress Intervention |
| DBT | Dialectical behaviour therapy |
| DDP | Dyadic developmental psychotherapy |
| DSM | Diagnostic and Statistical Manual of Mental Disorders |
| EBP | Evidence-based practice |
| EFT | Emotion-focused therapy |
| EMDR | Eye movement desensitisation and reprocessing |
| FBA | Functional behaviour assessment |
| ICD | International Classification of Diseases |
| IPT | Interpersonal psychotherapy |
| IPT-A | Interpersonal psychotherapy for adolescents |
| LGBTQ+ | Lesbian, gay, bisexual, transgender, queer/questioning and other sexual/gender minority identities |
| MBT | Mentalization-based treatment |
| MBT-A | Mentalization-based treatment for adolescents |
| MI | Motivational interviewing |
| MBCT | Mindfulness-based cognitive therapy |
| MBSR | Mindfulness-based stress reduction |
| NCTSN | National Child Traumatic Stress Network |
| NSSI | Non-suicidal self-injury |
| OCD | Obsessive-compulsive disorder |
| PTSD | Post-traumatic stress disorder |

| Acronym | Meaning |
|---|---|
| REBT | Rational emotive behaviour therapy |
| ROM | Routine outcome monitoring |
| SAMHSA | Substance Abuse and Mental Health Services Administration (United States) |
| SEND | Special educational needs and disabilities (terminology varies by jurisdiction) |
| SFBT | Solution-focused brief therapy |
| SUDS | Subjective units of distress scale |
| TF-CBT | Trauma-focused cognitive-behavioural therapy |

Acronyms change meaning across disciplines. Define them at first use in reports and avoid using them with families without explanation.

# I2. Course FAQ for Students

## Is this textbook a substitute for supervised clinical training?

No. It provides theory, formulation frameworks, teaching activities, and templates. It does not certify competence in EMDR, trauma processing, risk management, family therapy, testing, or any regulated intervention. Students require supervised practice and must follow institutional and professional requirements.

## Why does the book include theories with uneven evidence?

Professional literacy requires understanding influential models, their mechanisms, limitations, and contexts. Inclusion is not equal endorsement. Each chapter distinguishes evidence-supported components, plausible clinical traditions, contested claims, and practices requiring caution or specialist training.

## How should I choose a theory for a case?

Begin with the young person's goals, presenting pattern, development, culture, family and school contexts, risk, previous responses, and preferences. Identify mechanisms to change, select interventions with evidence and within competence, sequence them, explain the plan, and monitor outcomes. Do not begin by asking which school you personally prefer.

## What if theories conflict?

They often do. Use formulation to decide which level is most relevant now. For example, stabilisation may precede exposure; family accommodation may need to change while individual skills develop; cultural or institutional advocacy may be more urgent than intrapsychic work. Integration must be coherent and explainable.

## Can I use worksheets with real clients?

Templates require local adaptation, consent, safeguarding review, accessibility checks, and professional judgement. Some are teaching forms, not clinical records. Never place identifiable client information into

unsecured systems or external AI tools.

## How much should parents be told?

Parents need enough information to support safety, attendance, and agreed home or school actions. Adolescents also need a confidential therapeutic space. Explain at the outset what categories of information may be shared, obtain views where possible, follow law and safeguarding duties, and record agreements. Avoid verbatim reporting of ordinary session disclosures.

## What should I do when a young person does not want counselling?

Explore what referral means to them, what choices exist, what they fear, and whether attendance is mandated. Distinguish legal or safety requirements from negotiable goals. Motivational interviewing, practical changes, family work, school consultation, or a pause with a clear route back may be more ethical than forcing exploratory therapy.

## Is a diagnosis necessary?

Sometimes diagnosis guides treatment, eligibility, communication, and research. It is rarely sufficient for planning. Formulation adds development, relationships, culture, environment, strengths, mechanisms, and client meaning. Avoid diagnosing from brief vignettes or screening scores.

## How do I handle personal reactions to clients?

Notice them, do not automatically act on them, and use supervision. Emotional responses can reveal relational patterns, practitioner history, team pressure, fatigue, or bias. Record actions and decisions, not speculative claims about a young person's motives.

## What if I suspect abuse or exploitation?

Follow local safeguarding law and organisational procedure immediately. Counselling confidentiality does not override child protection duties. Tell the young person what will happen where safe, record accurately, avoid investigative questioning beyond your role, and seek designated safeguarding advice.

## How should culture be discussed?

Ask respectfully and specifically. Do not demand that a young person educate you about an entire group. Use cultural formulation, interpreters where needed, community resources chosen by the family, and self-reflection on practitioner power. Avoid stereotyping from this book's examples.

## Does affirmative LGBTQ+ practice mean agreeing with every request?

It means not pathologising identity, protecting safety and confidentiality, using chosen language, reducing stigma-related harm, and supporting informed exploration. Clinical, medical, safeguarding, and legal decisions still require individual assessment and local pathways.

## Why is there no faith-based content?

The course specification for this textbook requires professional counselling content without faith-related structures. Culturally responsive counsellors may still need to recognise when clients identify personal beliefs as important, but counsellors should not import religious teaching, ritual, or authority into this secular course framework.

### How should assessment scores be used?

As one source of evidence. Consider validity, norms, language, measurement error, functioning, context, and the young person's account. Scores can support screening, monitoring, and review; they cannot by themselves explain a life or determine worth.

### What makes a good reflective assignment?

Specific observation, separation of fact and inference, accurate theory, cultural and ethical analysis, recognition of uncertainty, and a concrete practice change. Personal disclosure is not the criterion.

### What should I read next after CAC. 701?

Follow the annotated pathways at the end of relevant chapters. Prioritise supervised practice, youth-specific evidence, developmental science, safeguarding, cultural humility, and outcome evaluation over collecting additional brand-name techniques.

## I3. Quick Course Navigation

- **Foundations:** Chapters 1–9
- **Humanistic and experiential:** Chapters 10–15
- **Cognitive and third-wave:** Chapters 16–22
- **Behavioural:** Chapters 23–29
- **Family and relational:** Chapters 30–36
- **Emotion, trauma, and interpersonal:** Chapters 37–43
- **Developmental, social, cultural:** Chapters 44–50
- **Integration and professional practice:** Chapters 51–58
- **Forms and worksheets:** Appendices A, F, G, H
- **Cases:** Appendix B
- **Comparisons and glossary:** Appendices C, D, I
- **Reading pathways:** Appendix E and chapter bibliographies

## Additional Student Questions

### How long should a formulation be?

Long enough to guide decisions and short enough to use. A one-page working formulation with linked mechanisms, strengths, risks, interventions, responsibilities, and review evidence is usually more useful

than a long biography.

## Should I tell a client my theoretical orientation?

Explain your approach in ordinary language when it affects what will happen, what is expected, and what choices exist. Technical labels are optional; informed participation is not.

## What if a technique works but I do not know why?

Monitor outcome and safety, consider common factors and context, discuss in supervision, and avoid claiming a mechanism the evidence does not support.

## What if parents ask for behaviour control?

Explore safety and family strain, explain developmental and ethical limits, assess function, and negotiate goals centred on wellbeing, relationships, learning, and participation rather than obedience alone.

## Can I combine counselling with advocacy?

Yes, where role and consent permit. Advocacy may address interpreters, disability access, bullying, housing, school adjustments, or service coordination. Record the young person's wishes and information-sharing basis.

## How do I know when to refer?

Refer when need exceeds competence, setting, or available intensity; when specialist assessment is required; when risk cannot be managed; or when another service is better matched. Continue appropriate support during transition.

## What should I do after a poor session?

Review promptly, consider rupture and practitioner contribution, check risk or follow-up needs, consult supervision, and plan repair. Do not wait passively for the next scheduled contact if safety or alliance requires action.

## How can I avoid cultural stereotyping?

Ask the individual and family, examine institutional context, use interpreters properly, and treat cultural knowledge as hypotheses to check rather than traits to assign.

## Are practice activities actual therapy?

No. Course role plays and worksheets are educational simulations. Real clinical use requires assessment, consent, competence, supervision, and adaptation.

## What is the most important habit to take from CAC. 701?

Keep returning to formulation: what is happening, for whom, in which contexts, maintained by what, protected by what, and what change would matter enough to test and review?

# I4. How to Study CAC. 701

Theory is learned through contrast, application, and revision. Reading a chapter once produces familiarity, not clinical judgement. A stronger study cycle has seven movements.

First, read the chapter overview and learning objectives before the detailed sections. Predict what the theory will say about a young person's difficulty. This prediction matters because it exposes the assumptions you bring from earlier training, family experience, school experience, or popular psychology.

Second, identify the theory's unit of analysis. Is the practitioner primarily watching behaviour, thought, emotion, relationship, family sequence, story, body state, culture, development, or institutions? Many disagreements between schools are disagreements about where to look first.

Third, translate every major concept into an observable clinical moment. If a term cannot be connected to something a young person says, does, avoids, feels, or experiences from adults, it has not yet become usable knowledge.

Fourth, construct two formulations of the same fictional case: one using the chapter's model and one using a contrasting model. State what each notices, what each misses, what each would do first, and what evidence would change your mind.

Fifth, practise one small skill in a fictional laboratory. Do not attempt to perform an entire therapy. Practise a contract, a reflection, a functional question, a scaling question, a family opening, a grounding invitation, or a repair statement. Small skills reveal assumptions quickly.

Sixth, interrogate development, culture, evidence, and power. Would this work with a younger child, an autistic adolescent, a young carer, a student learning the language of instruction, a young person in care, or a family facing material insecurity? What adaptation is required, and who should make it?

Seventh, write a short review after a delay. Retrieval after several days exposes what was merely recognised during reading. Use the chapter review questions without looking at the text, then return to correct and extend your answers.

## Reading primary sources

Primary texts matter because later summaries often flatten controversy. When reading a founder or major theorist, ask when the work was written, what clinical population it addressed, what methods were available, which claims were empirical, and which were philosophical. Historical importance does not settle present effectiveness.

Pair every classic with a contemporary review. Pair every outcome trial with an implementation question. Pair every manual with a cultural or developmental critique. This triangular reading habit prevents both uncritical adoption and fashionable dismissal.

## Preparing a case presentation

A strong presentation begins with the young person's own words, then context, then pattern, then formulation. It does not begin with a diagnostic label followed by evidence selected to support it. Include what you do not know. State who provided each piece of information. Identify whether family, school, and the young person agree about the problem.

End with a decision: What will be attempted first? What will not be attempted yet? Who else must act? What outcome and date will test the plan? What ethical issue requires supervision? A case presentation without a decision is a story; a case presentation with an unjustified decision is a risk.

## Working in a study group

Assign roles. One student summarises the theory, one challenges its evidence, one examines adolescent adaptation, one examines culture and power, and one converts the discussion into a treatment plan. Rotate roles across weeks so that critique does not become one person's identity and advocacy does not become uncritical loyalty.

Use fictional or fully anonymised material. Do not bring identifiable client details into peer study spaces. If a case resembles your own life or a current client too closely, step back and use supervision or personal support through appropriate channels.

## Preparing for assessment

Examiners reward comparison, application, and justification. When revising, create one-page maps for each theory: person, problem, change, counsellor role, techniques, development, culture, evidence, limits, and best-fit presentation. Then practise applying two maps to an unseen vignette under time limits.

Avoid memorising quotations detached from use. A smaller number of accurately understood concepts will outperform a large vocabulary used decoratively. In skills assessment, markers look for contracting, listening, pacing, safety, responsiveness, and reflection, not theatrical breakthroughs.

## A final professional habit

After every chapter, write one sentence beginning: “Because of this theory, I will now notice…” and one sentence beginning: “Because of its limits, I will be careful not to…” Keep those sentences. Across fifty-eight chapters they become a personal practice philosophy—specific enough to guide behaviour and humble enough to revise.

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